Elizabeth Tina Petruccio

Patient Health Summary, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Allergies

Clindamycin (Other (see comments))

Lactose (Other (see comments)) - Low Criticality
Lincosamides (Other (see comments), Unknown Reaction), Inactive

Medications

Advair HFA 115-21 mcg/actuation inhaler
Inhale 2 puffs 2 (two) times a day.

ammonium lactate (LAC-HYDRIN) 12 % lotion (Started 2/16/2024)
Apply topically 2 (two) times a day.

multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet (Started 2/17/2024)
Take 1 tablet by mouth daily.

apixaban (ELIQUIS) 5 mg tablet (Started 8/14/2024)
Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.
3 refills by 8/14/2025

gabapentin (NEURONTIN) 300 mg capsule (Started 9/18/2024)
Take 2 capsules (600 mg total) by mouth 3 (three) times a day.

benzonatate (TESSALON) 100 mg capsule (Started 9/23/2024)
Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.

predniSONE (DELTASONE) 20 mg tablet (Started 9/23/2024)
Take 2 tablets (40 mg total) by mouth daily with breakfast.

albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler (Started 10/15/2024)
INHALE 2 PUFFS EVERY 4 (FOUR) HOURS AS NEEDED FOR WHEEZING. CALOPTIMA MCAL ID 93931022D30310
3 refills by 10/15/2025

potassium chloride (MICRO-K) 10 mEq CR capsule (Started 11/2/2025)
TAKE 1 CAPSULE BY MOUTH EVERY DAY
3 refills by 11/2/2026

sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet (Started 11/2/2025)
TAKE 1 TABLET BY MOUTH TWICE A DAY
3 refills by 11/2/2026

furosemide (LASIX) 40 mg tablet (Started 11/3/2025)
TAKE 1 TABLET BY MOUTH EVERY DAY
1 refill by 11/3/2026

metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet (Started 11/4/2025)
TAKE 1 TABLET (25 MG TOTAL) BY MOUTH DAILY.

Active Problems

Problem	Noted Date	Diagnosed Date
Gallstones	02/18/2024	
Gall bladder pain	02/18/2024	
Pus collection in space between lung and chest wall	01/26/2024	
Acute osteomyelitis of cervical spine	01/26/2024	
Heart failure	01/13/2024	
Loculated pleural effusion	01/13/2024	
Atrial fibrillation (irregular heartbeat)	01/13/2024	
Respiratory insufficiency	01/12/2024	
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Resolved Problems

Problem	Noted Date	Diagnosed Date	Resolved Date
Severe sepsis	02/18/2024		03/24/2024

Immunizations

Tdap greater than or equal to 7yrs IM (Given 2/9/2024)

Social History

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Passive Smoke Exposure: Past				
Smokeless Tobacco: Never				

Tobacco Cessation: Counseling Given: Not Answered

Patient History	Answer	Date Recorded
Alcohol Use Status	Not Currently	04/18/2024
Oz/Week (Alcohol Misuse Male >= 9 OR Female >= 4.8)	Not on file	04/18/2024

Housing Stability	Answer	Date Recorded
What is your living situation today?	Do not have a stable place to live	01/19/2024

Food Insecurity	Answer	Date Recorded
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024

Transportation Needs	Answer	Date Recorded
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024

Pregnant	Comments
No	

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

Last Filed Vital Signs

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	104/68	06/23/2025 3:04 AM PDT	
Pulse	89	06/23/2025 3:15 AM PDT	
Temperature	36.6 °C (97.9 °F)	06/23/2025 3:04 AM PDT	
Respiratory Rate	21	06/23/2025 3:15 AM PDT	
Oxygen Saturation	96%	06/23/2025 3:15 AM PDT	
Inhaled Oxygen Concentration	-	-	
Weight	65.9 kg (145 lb 4.5 oz)	06/22/2025 10:53 PM PDT	
Height	167.6 cm (5' 6")	06/22/2025 10:16 PM PDT	
Body Mass Index	23.45	06/22/2025 10:16 PM PDT	

Procedures

Echocardiogram 2D With Definity Protocol As Needed (ECHOCARDIOGRAM 2D (WITH DEFINITY PROTOCOL/COLOR DOPPLER/SPECTRAL DOPPLER/STRAIN/3D PRN)) (Performed 9/18/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/12/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/12/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/11/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/11/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/9/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/9/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/8/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/7/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/7/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/6/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/6/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/5/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/4/2024)
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CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/3/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/2/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/1/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 2/1/2024)
RESPIRATORY THERAPY EVAL AND TREAT PROTOCOL (Performed 1/31/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 1/31/2024)
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CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 1/15/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 1/14/2024)
EXTUBATION (Performed 1/14/2024)
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 1/14/2024)
VENTILATOR (Performed 1/13/2024)

TUBE THORACOSTOMY INCLUDES WATER SEAL (Performed 1/13/2024)
Performed for Loculated pleural effusion

Echocardiogram 2D With Definity Protocol As Needed (ECHOCARDIOGRAM 2D (WITH DEFINITY PROTOCOL/COLOR DOPPLER/SPECTRAL DOPPLER/STRAIN/3D PRN)) (Performed 1/13/2024)

CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 1/13/2024)

CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 1/13/2024)

VENTILATOR (Performed 1/13/2024)

VENTILATOR (Performed 1/13/2024)

CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES) (Performed 1/13/2024)

Results

CT lower extremity left without contrast (CT LOWER EXTREMITY LEFT WITHOUT CONTRAST) - Final result (06/23/2025 12:30 AM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities	Left	Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			06/23/2025 12:38 AM PDT	

Impressions

06/23/2025 10:12 AM PDT
IMPRESSION:
Large subcutaneous hematoma in the left mid calf measuring up to 15.8 cm. No acute osseous abnormality.

release

Narrative

06/23/2025 10:12 AM PDT
EXAMINATION: CT LOWER EXTREMITY LEFT WITHOUT CONTRAST

DATE/TIME: 6/23/2025 12:30 AM

HISTORY: L lower extremity pain/swelling

COMPARISON: Radiographs from 6/22/2025

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:
EXTREMITY: Left lower leg
Mild diffuse subcutaneous edema.
There is a large blood attenuation fluid collection in the subcutaneous soft tissues of the left mid calf measuring approximately 4.3 x 8.1 x 15.8 cm
No acute fracture

EXPOSURE:
This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 2; Series: 7; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 9; DLP: 202
Exposure: 1; Series: 4; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 8; DLP: 529

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

Procedure Note

Jeffrey Rudie, MD, PhD - 06/23/2025

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: CT LOWER EXTREMITY LEFT WITHOUT CONTRAST

DATE/TIME: 6/23/2025 12:30 AM

HISTORY: L lower extremity pain/swelling

COMPARISON: Radiographs from 6/22/2025

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

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EXTREMITY: Left lower leg
Mild diffuse subcutaneous edema.
There is a large blood attenuation fluid collection in the subcutaneous soft tissues of the left mid calf measuring approximately 4.3 x 8.1 x 15.8 cm
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Exposure: 2; Series: 7; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 9; DLP: 202
Exposure: 1; Series: 4; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 8; DLP: 529

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

IMPRESSION:
IMPRESSION:
Large subcutaneous hematoma in the left mid calf measuring up to 15.8 cm. No acute osseous abnormality.

release

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	IMG CT PROCEDURES	Final Result

X-ray tibia and fibula left 1 or 2 views (X-RAY TIBIA AND FIBULA LEFT 1 OR 2 VIEWS) - Final result (06/22/2025 11:03 PM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities, Lower Leg	Left	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			06/23/2025 8:05 AM PDT	

Impressions

06/23/2025 8:06 AM PDT
IMPRESSION: No fracture or periosteal process appreciated.
2. Soft tissue lucency in the calf consistent with fluid collection

release

Narrative

06/23/2025 8:06 AM PDT

Narrative

EXAMINATION: X-RAY TIBIA AND FIBULA LEFT 1 OR 2 VIEWS

DATE/TIME:6/22/2025 11:03 PM

HISTORY: LLE pain

COMPARISON: None.

FINDINGS:

BONE: No apparent bony abnormality.
SOFT TISSUE: Soft tissue lucency in the posterior calf region
OTHER: Negative.

Procedure Note

Trevor Nelson, MD - 06/23/2025
Formatting of this note might be different from the original.
EXAMINATION: X-RAY TIBIA AND FIBULA LEFT 1 OR 2 VIEWS

DATE/TIME:6/22/2025 11:03 PM

HISTORY: LLE pain

COMPARISON: None.

FINDINGS:

BONE: No apparent bony abnormality.
SOFT TISSUE: Soft tissue lucency in the posterior calf region
OTHER: Negative.

IMPRESSION:
IMPRESSION: No fracture or periosteal process appreciated.
2. Soft tissue lucency in the calf consistent with fluid collection

release

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CULTURE, BLOOD (BLOOD CULTURE) - Final result (06/22/2025 10:52 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			06/28/2025 12:01 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of left upper limb / Unknown	Venipuncture / Unknown	06/22/2025 10:52 PM PDT	06/22/2025 11:03 PM PDT

Narrative

MERCY LAB CV - 06/28/2025 12:01 AM PDT
Reference range: No growth

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	7.7	3.4 - 11.0 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
RBC	2.70	3.98 - 5.25 M/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
HGB	8.5	11.9 - 15.3 g/dL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
HCT	26.2	37.3 - 46.7 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MCV	97	81 - 100 fL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MCH	32	26 - 33 pg		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MCHC	32.4	31.0 - 36.0 g/dL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
RDW	12.8	11.7 - 14.9 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Platelet Count	387	150 - 425 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MPV	8.5	9.0 - 12.8 fL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Neutrophils %	56.4	38.0 - 74.0 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
IG%	0.5	0.0 - 1.2 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Lymphocytes %	27.8	16.0 - 48.0 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Monocytes %	8.5	4.9 - 12.5 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Eosinophils %	6.1	0.0 - 9.5 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Neutrophils	4.33	1.50 - 7.40 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	2.13	0.90 - 3.10 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Monocytes	0.65	0.26 - 0.87 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Eosinophils	0.47	0.00 - 0.51 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Preliminary Abs Neut	4.33	1.50 - 7.40 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			06/28/2025 12:01 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous line / Unknown		06/22/2025 10:49 PM PDT	06/22/2025 11:03 PM PDT

Narrative

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MERCY LAB CV - 06/28/2025 12:01 AM PDT
Reference range: No growth
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Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

APTT (ACTIVATED PTT) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
aPTT	33	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds		06/22/2025 11:32 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTIME-INR (PROTHROMBIN TIME-INR) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protime	10.4	10.0 - 13.1 Seconds		06/22/2025 11:32 PM PDT	MERCY LAB CV	
INR	0.9	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation		06/22/2025 11:32 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTIC ACID, SERUM (LACTIC ACID, VENOUS) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Lactic Acid, Venous	1.20	0.70 - 2.10 mmol/L		06/22/2025 11:13 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:00 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				06/22/2025 11:24 PM PDT	MERCY LAB CV	
Glucose	87	70 - 125 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Sodium	142	137 - 145 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Potassium	4.7	3.5 - 5.1 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Chloride	106	98 - 107 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
CO2, Total	24	22 - 30 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Urea Nitrogen	48	7 - 17 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Creatinine	2.10	0.50 - 1.00 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	
CKD-EPI eGFR 2021	26	>60 mL/min/1.73m*2		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Anion Gap	12	6 - 14 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Osmolality Calculated	306	280 - 305 mOsm/kg H20		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

MERCY LAB CV - 06/22/2025 11:24 PM PDT

Narrative

```
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (12/24/2024 2:46 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.053	<=0.034 ng/mL		12/24/2024 3:20 AM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population.  This patient's result is suspicious for myocardial injury. However,
serial measurements may be necessary to confirm or exclude the diagnosis of acute myocardial infarction.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 2:46 AM PST	12/24/2024 2:50 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (12/24/2024 1:44 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/24/2024 8:10 AM PST	

Impressions

```
12/24/2024 8:50 AM PST
IMPRESSION: Similar exam with no acute cardiac or pulmonary disease appreciated


release
```

Narrative

12/24/2024 8:50 AM PST

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/24/2024 1:44 AM

HISTORY: chest pain

COMPARISON: 9/23/2024

FINDINGS:

LUNGS: Normal. No infiltrates or other abnormalities.
VASCULATURE: Normal. Unremarkable pulmonary vasculature.
CARDIAC: Normal. No cardiac silhouette abnormality.
MEDIASTINUM: Normal. No visible mass or adenopathy.
PLEURA: Normal. No effusion or pleural thickening.
BONES: Normal. No fracture or visible bony lesion.
OTHER: Negative.

Procedure Note

Trevor Nelson, MD - 12/24/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/24/2024 1:44 AM

HISTORY: chest pain

COMPARISON: 9/23/2024

FINDINGS:

LUNGS: Normal. No infiltrates or other abnormalities.
VASCULATURE: Normal. Unremarkable pulmonary vasculature.
CARDIAC: Normal. No cardiac silhouette abnormality.
MEDIASTINUM: Normal. No visible mass or adenopathy.
PLEURA: Normal. No effusion or pleural thickening.
BONES: Normal. No fracture or visible bony lesion.
OTHER: Negative.

IMPRESSION:
IMPRESSION: Similar exam with no acute cardiac or pulmonary disease appreciated

release

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	1,560	Refer to the reference range comment. pg/mL		12/24/2024 1:31 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

NT-proBNP reference range: <=125 pg/mL

NT-proBNP testing aids in the diagnosis or exclusion of heart failure (HF) when used in conjunction with clinical history, symptoms, exam findings, and additional imaging and laboratory studies.

Emergency Department:
The ICON-RELOADED study provided validation of cutoff values for NT-proBNP in patients presenting with dyspnea in the Emergency Department (1).
In this setting NT-proBNP level <300 pg/mL has a high negative predictive value for HF. For values above 300 pg/mL, age-specific cutoffs provide additional decision limit categories:

Result.....Age.....Interpretation
<300.....All.....Negative: Heart Failure Unlikely
=300 to <450.....22-<50..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <900.....50-<75..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <1800....>=75....Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=>450.....22-<50..Positive: Heart Failure Likely
=>900.....50-<75..Positive: Heart Failure Likely
=>1800.....>=75....Positive: Heart Failure Likely

* Natriuretic peptides values in the gray zones may be caused by conditions other than heart failure. Acute coronary syndrome,pulmonary embolism,pulmonary hypertension,sepsis,stroke,and renal dysfunction will elevate NT- proBNP levels. Obesity, flash pulmonary edema,cardiac tamponade,and pericardial constriction are conditions associated with reduced NT-proBNP(2).

Ambulatory Clinics:
For patients presenting to outpatient clinics with suspicion of HF, the optimal clinical utility of NT-proBNP is to
exclude heart failure.For this clinical setting Ortho Clinical Diagnostics recommends NT-proBNP results be interpreted
as in the table below(2):

Result.....Age.....Interpretation
<125.....All.....Negative: Heart Failure Unlikely
>=125.....All.....Consider Heart Failure as well as other causes* of NT-proBNP elevation.

* Natriuretic peptides value elevations could also be caused by conditions other than heart failure such as acute
coronary syndrome, pulmonary embolism, pulmonary hypertension, sepsis, stroke, and renal dysfunction (2).

References:
1. Terminal Pro-B-Type Natriuretic Peptide in the Emergency Department: The ICON-RELOADED Study. J Am Coll Cardiol. 2018 Mar 20;71(11):1191-1200. doi: 10.1016/j.jacc.2018.01.021. PMID: 29544601.

2. NBNP2 Instructions For Use. Ortho Clinical Diagnostics

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	6.4	3.4 - 11.0 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
RBC	3.16	3.98 - 5.25 M/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
HGB	9.5	11.9 - 15.3 g/dL		12/24/2024 1:05 AM PST	MERCY LAB CV	
HCT	31.7	37.3 - 46.7 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		12/24/2024 1:05 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		12/24/2024 1:05 AM PST	MERCY LAB CV	
MCHC	30.0	31.0 - 36.0 g/dL		12/24/2024 1:05 AM PST	MERCY LAB CV	
RDW	14.6	11.7 - 14.9 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Platelet Count	411	150 - 425 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
MPV	8.1	9.0 - 12.8 fL		12/24/2024 1:05 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		12/24/2024 1:05 AM PST	MERCY LAB CV	
Neutrophils %	52.3	38.0 - 74.0 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
IG%	4.1	0.0 - 1.2 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Lymphocytes %	31.8	16.0 - 48.0 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Monocytes %	6.3	4.9 - 12.5 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Eosinophils %	4.9	0.0 - 9.5 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Basophils %	0.6	0.0 - 1.6 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.32	1.50 - 7.40 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
IG#	0.26	0.00 - 0.10 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.02	0.90 - 3.10 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Monocytes	0.40	0.26 - 0.87 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.31	0.00 - 0.51 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Preliminary Abs Neut	3.32	1.50 - 7.40 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.041	<=0.034 ng/mL		12/24/2024 1:31 AM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population.  This patient's result is suspicious for myocardial injury. However,
serial measurements may be necessary to confirm or exclude the diagnosis of acute myocardial infarction.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/24/2024 1:18 AM PST	MERCY LAB CV	
Glucose	91	70 - 125 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	
Sodium	141	137 - 145 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Urea Nitrogen	36	7 - 17 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	
Creatinine	1.25	0.50 - 1.00 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	49	>60 mL/min/1.73m*2		12/24/2024 1:18 AM PST	MERCY LAB CV	
Anion Gap	8	6 - 14 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Osmolality Calculated	300	280 - 305 mOsm/kg H20		12/24/2024 1:18 AM PST	MERCY LAB CV	
Calcium	9.3	8.4 - 10.2 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

MERCY LAB CV - 12/24/2024 1:18 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (12/24/2024 12:50 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	86	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	86	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	178	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	100	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	410	ms			SCRIPPS RADIOLOGY	
QTC	490	ms			SCRIPPS RADIOLOGY	
P AXIS	59	degrees			SCRIPPS RADIOLOGY	
R AXIS	16	degrees			SCRIPPS RADIOLOGY	
T AXIS	91	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/24/2024 12:50 AM PST	12/24/2024 10:12 AM PST

Impressions

SCRIPPS RADIOLOGY - 12/24/2024 10:12 AM PST
Normal sinus rhythm with sinus arrhythmia
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific ST and T wave abnormality
Prolonged QT
Abnormal ECG
When compared with ECG of 23-Sep-2024 10:49,
aberrant conduction is no longer present

Narrative

This result has an attachment that is not available.

Procedure Note

Douglas Ballesteros, MD - 12/24/2024
Formatting of this note might be different from the original.
IMPRESSION:
Normal sinus rhythm with sinus arrhythmia
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific ST and T wave abnormality
Prolonged QT
Abnormal ECG
When compared with ECG of 23-Sep-2024 10:49,
aberrant conduction is no longer present

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

CULTURE, BLOOD (BLOOD CULTURE) - Final result (09/23/2024 4:19 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			09/28/2024 5:07 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of left upper limb / Unknown	Venipuncture / Unknown	09/23/2024 4:19 PM PDT	09/23/2024 4:21 PM PDT

Narrative

MERCY LAB CV - 09/28/2024 5:07 PM PDT
Reference range: No growth

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (09/23/2024 4:16 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			09/28/2024 5:07 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of left upper limb / Unknown	Venipuncture / Unknown	09/23/2024 4:16 PM PDT	09/23/2024 4:21 PM PDT

Narrative

MERCY LAB CV - 09/28/2024 5:07 PM PDT
Reference range: No growth

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray chest 2 views with pa/ap and lateral (X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL) - Final result (09/23/2024 11:47 AM PDT)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			09/23/2024 11:48 AM PDT	

Impressions

09/23/2024 11:49 AM PDT
IMPRESSION: Decreased right pleural effusion in comparison to 1/29/2024. No new infiltrate.

release

Narrative

09/23/2024 11:49 AM PDT

Narrative

EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 9/23/2024 11:47 AM

HISTORY: productive cough

COMPARISON: 1/29/2024

FINDINGS:

The cardiomediastinal silhouette is normal in size and contour.

Decreased right pleural effusion in comparison to 1/29/2024.

No evidence of fracture or suspicious osseous lesion.

Procedure Note

Brian Shore, MD - 09/23/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 9/23/2024 11:47 AM

HISTORY: productive cough

COMPARISON: 1/29/2024

FINDINGS:

The cardiomediastinal silhouette is normal in size and contour.

Decreased right pleural effusion in comparison to 1/29/2024.

No evidence of fracture or suspicious osseous lesion.

IMPRESSION:
IMPRESSION: Decreased right pleural effusion in comparison to 1/29/2024. No new infiltrate.

release

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	3,460	Refer to the reference range comment. pg/mL		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

NT-proBNP reference range: <=125 pg/mL

NT-proBNP testing aids in the diagnosis or exclusion of heart failure (HF) when used in conjunction with clinical history, symptoms, exam findings, and additional imaging and laboratory studies.

Emergency Department:
The ICON-RELOADED study provided validation of cutoff values for NT-proBNP in patients presenting with dyspnea in the Emergency Department (1).
In this setting NT-proBNP level <300 pg/mL has a high negative predictive value for HF. For values above 300 pg/mL, age-specific cutoffs provide additional decision limit categories:

Result.....Age.....Interpretation
<300.....All.....Negative: Heart Failure Unlikely
=300 to <450.....22-<50..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <900.....50-<75..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <1800....>=75....Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=>450.....22-<50..Positive: Heart Failure Likely
=>900.....50-<75..Positive: Heart Failure Likely
=>1800.....>=75....Positive: Heart Failure Likely

* Natriuretic peptides values in the gray zones may be caused by conditions other than heart failure. Acute coronary syndrome,pulmonary embolism,pulmonary hypertension,sepsis,stroke,and renal dysfunction will elevate NT- proBNP levels. Obesity, flash pulmonary edema,cardiac tamponade,and pericardial constriction are conditions associated with reduced NT-proBNP(2).

Ambulatory Clinics:
For patients presenting to outpatient clinics with suspicion of HF, the optimal clinical utility of NT-proBNP is to
exclude heart failure.For this clinical setting Ortho Clinical Diagnostics recommends NT-proBNP results be interpreted
as in the table below(2):

Result.....Age.....Interpretation
<125.....All.....Negative: Heart Failure Unlikely
>=125.....All.....Consider Heart Failure as well as other causes* of NT-proBNP elevation.

* Natriuretic peptides value elevations could also be caused by conditions other than heart failure such as acute
coronary syndrome, pulmonary embolism, pulmonary hypertension, sepsis, stroke, and renal dysfunction (2).

References:
1. Terminal Pro-B-Type Natriuretic Peptide in the Emergency Department: The ICON-RELOADED Study. J Am Coll Cardiol. 2018 Mar 20;71(11):1191-1200. doi: 10.1016/j.jacc.2018.01.021. PMID: 29544601.

2. NBNP2 Instructions For Use. Ortho Clinical Diagnostics

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROCALCITONIN - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Procalcitonin	0.36	0.03 - 0.08 ng/mL		09/23/2024 12:05 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 12:05 PM PDT
Procalcitonin (PCT) is a dynamic biomarker with value in guiding antibiotic management in select clinical settings. PCT values are most useful when trends are analyzed. Decisions on antibiotic use should not be based solely on PCT level. Consider repeating PCT no more than daily in ICU/sepsis patients, and every 2 days for LRTI in order to discontinue antibiotics.

Suspected Lower Respiratory Tract Infection (LRTI):
0.1-0.25 ng/mL - low likelihood for bacterial infection, antibiotics discouraged
>0.25 ng/mL - increased likelihood for bacterial infection.

Suspected Sepsis
0.1-0.5 ng/mL - low likelihood for sepsis, antibiotics discouraged.
>0.5 ng/mL - increased likelihood of sepsis.
>2.0 ng/mL - high risk of sepsis/septic shock.

Consider discontinuation of antibiotics when PCT <0.25 (LRTI) or <0.5 (sepsis), or 80% reduction from baseline.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	8.8	3.4 - 11.0 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
RBC	3.96	3.98 - 5.25 M/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
HGB	11.5	11.9 - 15.3 g/dL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
HCT	37.7	37.3 - 46.7 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MCV	95	81 - 100 fL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MCH	29	26 - 33 pg		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MCHC	30.5	31.0 - 36.0 g/dL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
RDW	14.4	11.7 - 14.9 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	336	150 - 425 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MPV	8.7	9.0 - 12.8 fL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Neutrophils %	78.4	38.0 - 74.0 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
IG%	0.3	0.0 - 1.2 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Lymphocytes %	16.9	16.0 - 48.0 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Monocytes %	4.2	4.9 - 12.5 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Eosinophils %	0.0	0.0 - 9.5 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Neutrophils	6.89	1.50 - 7.40 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Lymphocytes	1.49	0.90 - 3.10 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Monocytes	0.37	0.26 - 0.87 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Eosinophils	0.00	0.00 - 0.51 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Basophils	0.02	0.00 - 0.09 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Preliminary Abs Neut	6.89	1.50 - 7.40 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	<0.012	<=0.034 ng/mL		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population. This patient's result is considered normal. However, clinical
correlation is suggested to determine whether serial measurements are necessary for further evaluation.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

APTT (ACTIVATED PTT) - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
aPTT	36	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:25 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTIME-INR (PROTHROMBIN TIME-INR) - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protime	12.6	10.0 - 13.1 Seconds		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
INR	1.1	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:25 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	11.9	<10.0 mg/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				09/23/2024 11:54 AM PDT	MERCY LAB CV	
Glucose	118	70 - 125 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	102	98 - 107 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Urea Nitrogen	23	7 - 17 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Creatinine	0.81	0.50 - 1.00 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
CKD-EPI eGFR 2021	83	>60 mL/min/1.73m*2		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Anion Gap	7	6 - 14 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Osmolality Calculated	291	280 - 305 mOsm/kg H20		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Calcium	8.9	8.4 - 10.2 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Protein, total	7.9	6.3 - 8.2 g/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Alkaline Phosphatase	86	38 - 126 Units/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
AST	29	14 - 36 Units/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
ALT	15	<=34 Units/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Bilirubin, Total	0.4	0.2 - 1.3 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 11:54 AM PDT

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COVID-19, RAPID COMBO (COVID-19, RAPID (PCR/NAAT, COMBO)) - Final result (09/23/2024 11:16 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected		09/23/2024 12:32 PM PDT	MERCY LAB CV	

Comment:

A Detected result is considered a positive result for that virus.
A positive SARS-CoV-2 result indicates that RNA from SARS-CoV-2 was detected and that the patient is considered infected and presumed contagious. A positive Influenza A or B result indicates that RNA from Influenza A or B, respectively, was detected and that the patient is considered infected and presumed contagious.

A Not Detected (negative) test result means that RNA was not present in the specimen above the limit of detection. However, it does not rule out the possibility of infection and should not be used as the sole basis for patient management decisions. Test parameters for asymptomatic patients have not been evaluated.

These results have been reported to the San Diego Dept. of Health Services as required by Title 17, California Code of regulations, Section 2500.
Test information for patients can be found at the following link: https://www.fda.gov/media/142192/download
Test information for providers can be found at the following link: https://www.fda.gov/media/142191/download

This SARS-CoV-2 Real Time RT-PCR test was performed under an Emergency Use Authorization granted by the FDA for the Roche Liat.

This test meets Covid-19 testing requirements for school and business testing criteria and is not an antibody or antigen test.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		09/23/2024 11:16 AM PDT	09/23/2024 11:23 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

INFLUENZA A/B COMBO - Final result (09/23/2024 11:16 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Influenza A PCR	Not Detected	Not Detected		09/23/2024 12:32 PM PDT	MERCY LAB CV	
Influenza B PCR	Not Detected	Not Detected		09/23/2024 12:32 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		09/23/2024 11:16 AM PDT	09/23/2024 11:23 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 12:32 PM PDT
These results have been reported to the San Diego Department of Health Services as required by Title 17, California Code of Regulations, Section 2505.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RSV BY PCR (RSV (PCR/NAAT)) - Final result (09/23/2024 11:15 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
RSV by PCR	Not Detected	Not Detected		09/23/2024 12:33 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
NP Swab	Nasopharyngeal structure / Unknown		09/23/2024 11:15 AM PDT	09/23/2024 11:23 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 12:33 PM PDT
These results have been reported to the San Diego Department of Health Services as required by Title 17, California Code of regulations, Section 2505.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (09/23/2024 10:49 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	101	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	101	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	158	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	92	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	370	ms			SCRIPPS RADIOLOGY	
QTC	479	ms			SCRIPPS RADIOLOGY	
P AXIS	60	degrees			SCRIPPS RADIOLOGY	
R AXIS	14	degrees			SCRIPPS RADIOLOGY	
T AXIS	123	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			09/23/2024 10:49 AM PDT	09/23/2024 2:20 PM PDT

Impressions

SCRIPPS RADIOLOGY - 09/23/2024 2:20 PM PDT
Sinus tachycardia with premature atrial complexes with aberrant conduction
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific T wave abnormality
Abnormal ECG
When compared with ECG of 11-Feb-2024 10:50,
aberrant conduction is now present
Nonspecific T wave abnormality no longer evident in Inferior leads
T wave inversion less evident in Lateral leads

Narrative

This result has an attachment that is not available.

Procedure Note

Mark Hernandez, MD - 09/23/2024
Formatting of this note might be different from the original.
IMPRESSION:
Sinus tachycardia with premature atrial complexes with aberrant conduction
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific T wave abnormality
Abnormal ECG
When compared with ECG of 11-Feb-2024 10:50,
aberrant conduction is now present
Nonspecific T wave abnormality no longer evident in Inferior leads
T wave inversion less evident in Lateral leads

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

Echocardiogram 2D With Definity Protocol As Needed (ECHOCARDIOGRAM 2D WITH IMAGE ENHANCEMENT AGENT AS NEEDED) - Final result (09/18/2024 2:54 PM PDT)

Anatomical Region	Laterality	Modality
		Ultrasound

Narrative

This result has an attachment that is not available.

Authorizing Provider	Result Type	Result Status
Jerrold Glassman MD	CV ECHO PROCEDURES	Final Result

12-LEAD ECG - Final result (08/14/2024 4:20 PM PDT)

Narrative

This result has an attachment that is not available.

Authorizing Provider	Result Type	Result Status
Jerrold Glassman MD	ECG ORDERABLES	Final Result

Nuc Med hepatobiliary sys without pharm including gallbladder (NUC MED HEPATOBILIARY SYSTEM WITHOUT PHARMACOLOGY) - Final result (02/16/2024 3:20 PM PST)

Anatomical Region	Laterality	Modality
		Nuclear Medicine

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/16/2024 3:16 PM PST	

Impressions

02/16/2024 3:59 PM PST

IMPRESSION: Normal gallbladder visualization. There is no evidence of acute cholecystitis or biliary ductal dilation.

release

Narrative

02/16/2024 3:59 PM PST

EXAMINATION: NUC MED HEPATOBILIARY SYSTEM WITHOUT PHARMACOLOGY

DATE/TIME: 2/16/2024 1:55 PM

HISTORY: RUQ abdominal pain, biliary disease suspected, US nondiagnostic

COMPARISON: Right upper quadrant ultrasound 2/15/2024

TECHNIQUE: Radiopharmaceutical was injected intravenously and sequential static biliary tract imaging was performed.

PHARMACEUTICAL: 5.3 mCi Tc-99m biliary IDA derivative

FINDINGS:
BILIARY DUCTS: Normal radioisotopic biliary excretion.
GALLBLADDER: Normal gallbladder visualization beginning at 25 minutes with no evidence of cystic duct obstruction.
INTESTINE: Normal progression of tracer into the small bowel seen beginning at 35 minutes. No evidence of common biliary ductal obstruction.
OTHER: None.

Procedure Note

Lida Chaipat, MD - 02/16/2024

Formatting of this note might be different from the original.

EXAMINATION: NUC MED HEPATOBILIARY SYSTEM WITHOUT PHARMACOLOGY

DATE/TIME: 2/16/2024 1:55 PM

HISTORY: RUQ abdominal pain, biliary disease suspected, US nondiagnostic

COMPARISON: Right upper quadrant ultrasound 2/15/2024

TECHNIQUE: Radiopharmaceutical was injected intravenously and sequential static biliary tract imaging was performed.

PHARMACEUTICAL: 5.3 mCi Tc-99m biliary IDA derivative

FINDINGS:
BILIARY DUCTS: Normal radioisotopic biliary excretion.
GALLBLADDER: Normal gallbladder visualization beginning at 25 minutes with no evidence of cystic duct obstruction.
INTESTINE: Normal progression of tracer into the small bowel seen beginning at 35 minutes. No evidence of common biliary ductal obstruction.
OTHER: None.

IMPRESSION:
IMPRESSION: Normal gallbladder visualization. There is no evidence of acute cholecystitis or biliary ductal dilation.

release

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	IMG NM PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (02/16/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.0	3.4 - 11.0 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
RBC	3.21	3.98 - 5.25 M/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
HGB	9.7	11.9 - 15.3 g/dL		02/16/2024 6:36 AM PST	MERCY LAB CV	
HCT	32.3	37.3 - 46.7 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/16/2024 6:36 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		02/16/2024 6:36 AM PST	MERCY LAB CV	
MCHC	30.0	31.0 - 36.0 g/dL		02/16/2024 6:36 AM PST	MERCY LAB CV	
RDW	13.4	11.7 - 14.9 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Platelet Count	305	150 - 425 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
MPV	8.4	9.0 - 12.8 fL		02/16/2024 6:36 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/16/2024 6:36 AM PST	MERCY LAB CV	
Neutrophils %	38.2	38.0 - 74.0 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
IG%	0.2	0.0 - 1.2 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Lymphocytes %	45.0	16.0 - 48.0 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Monocytes %	8.2	4.9 - 12.5 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Eosinophils %	7.4	0.0 - 9.5 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Neutrophils	1.91	1.50 - 7.40 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
IG#	0.01	0.00 - 0.10 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.25	0.90 - 3.10 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Monocytes	0.41	0.26 - 0.87 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.37	0.00 - 0.51 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Preliminary Abs Neut	1.91	1.50 - 7.40 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/16/2024 5:45 AM PST	02/16/2024 6:02 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/16/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/16/2024 6:24 AM PST	MERCY LAB CV	
Glucose	113	70 - 125 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Sodium	137	137 - 145 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Urea Nitrogen	23	7 - 17 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		02/16/2024 6:24 AM PST	MERCY LAB CV	
Anion Gap	6	6 - 14 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality Calculated	288	280 - 305 mOsm/kg H20		02/16/2024 6:24 AM PST	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Protein, total	8.4	6.3 - 8.2 g/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Alkaline Phosphatase	80	38 - 126 Units/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
AST	38	14 - 36 Units/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
ALT	26	<=34 Units/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/16/2024 5:45 AM PST	02/16/2024 6:02 AM PST

Narrative

MERCY LAB CV - 02/16/2024 6:24 AM PST

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for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

Ultrasound abdominal limited gallbladder (ULTRASOUND ABDOMEN LIMITED GALLBLADDER) - Final result (02/15/2024 6:30 PM PST)

Anatomical Region	Laterality	Modality
Body		Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/15/2024 6:58 PM PST	

Impressions

02/15/2024 7:01 PM PST

Impressions

IMPRESSION:
1. Tumefactive sludge favored over nonshadowing new gallstones compared to negative gallbladder study of 1/13/2024.
2. Slight gallbladder wall thickening without pericholecystic edema or sonographic Murphy sign not so pronounced to confirm acute cholecystitis. Nuclear medicine biliary scan can clarify if additional imaging indicated clinically.
3. Borderline hepatomegaly with slightly echogenic dense sonographic pattern suspicious for mild hepatic steatosis. No surface contour abnormality, or ascites to suggest cirrhosis. No visible lesion to suggest infiltrative malignancy.

release

Narrative

02/15/2024 7:01 PM PST
EXAMINATION: ULTRASOUND ABDOMEN LIMITED GALLBLADDER

DATE/TIME: 2/15/2024 6:30 PM

HISTORY: RUQ pain

ADDITIONAL CLINICAL INFORMATION: CHF.

COMPARISON: 1/13/2024.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: The echogenicity of the liver is increased. Liver is mildly enlarged at 17.4 cm cephalocaudad reaching well below the lower pole the right kidney. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
GALLBLADDER: The gallbladder now contains echogenic debris, nonshadowing. This most likely is related to tumefactive sludge rather than nonshadowing gallstones. Filled biliary interval dilatation of the common bile duct to 8.2 mm without visible calcified common duct stone or mass. No intra paddock duct dilatation.
BILIARY: No ductal dilatation.
PANCREAS: Visualized pancreatic head and body negative for focal mass or ductal dilatation.
RIGHT KIDNEY: No visible solid mass, calculus or hydronephrosis.
AORTA/VASC: No visible aneurysm.
IVC: The visible IVC is patent.
OTHER: No free fluid.

The following measurements were obtained:
AORTA PROXIMAL: 2.1 cm
AORTA DISTAL: 1.5 cm
LIVER: 17.4 cm
GALLBLADDER WALL: 3.2 mm
CBD: 8.2 mm
RT KIDNEY: 10.4 cm x 4.4 cm x 5.3 cm

Procedure Note

Kris Vanlom, MD - 02/15/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: ULTRASOUND ABDOMEN LIMITED GALLBLADDER

DATE/TIME: 2/15/2024 6:30 PM

HISTORY: RUQ pain

ADDITIONAL CLINICAL INFORMATION: CHF.

COMPARISON: 1/13/2024.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: The echogenicity of the liver is increased. Liver is mildly enlarged at 17.4 cm cephalocaudad reaching well below the lower pole the right kidney. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
GALLBLADDER: The gallbladder now contains echogenic debris, nonshadowing. This most likely is related to tumefactive sludge rather than nonshadowing gallstones. Filled biliary interval dilatation of the common bile duct to 8.2 mm without visible calcified common duct stone or mass. No intra paddock duct dilatation.
BILIARY: No ductal dilatation.
PANCREAS: Visualized pancreatic head and body negative for focal mass or ductal dilatation.
RIGHT KIDNEY: No visible solid mass, calculus or hydronephrosis.
AORTA/VASC: No visible aneurysm.
IVC: The visible IVC is patent.
OTHER: No free fluid.

The following measurements were obtained:
AORTA PROXIMAL: 2.1 cm
AORTA DISTAL: 1.5 cm
LIVER: 17.4 cm
GALLBLADDER WALL: 3.2 mm
CBD: 8.2 mm
RT KIDNEY: 10.4 cm x 4.4 cm x 5.3 cm

IMPRESSION:
IMPRESSION:
1. Tumefactive sludge favored over nonshadowing new gallstones compared to negative gallbladder study of 1/13/2024.
2. Slight gallbladder wall thickening without pericholecystic edema or sonographic Murphy sign not so pronounced to confirm acute cholecystitis. Nuclear medicine biliary scan can clarify if additional imaging indicated clinically.
3. Borderline hepatomegaly with slightly echogenic dense sonographic pattern suspicious for mild hepatic steatosis. No surface contour abnormality, or ascites to suggest cirrhosis. No visible lesion to suggest infiltrative malignancy.

release

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	IMG US PROCEDURES	Final Result

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (02/14/2024 11:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	111	<=30 mm/hr		02/14/2024 1:44 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/14/2024 11:11 AM PST	02/14/2024 11:27 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (02/14/2024 11:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	<5.0	<10.0 mg/L		02/14/2024 11:45 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/14/2024 11:11 AM PST	02/14/2024 11:24 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/14/2024 5:47 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	104	70 - 125 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Sodium	137	137 - 145 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
CO2, Total	33	22 - 30 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Creatinine	0.96	0.50 - 1.00 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	68	>60 mL/min/1.73m*2		02/14/2024 6:24 AM PST	MERCY LAB CV	
Anion Gap	3	6 - 14 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality Calculated	287	280 - 305 mOsm/kg H20		02/14/2024 6:24 AM PST	MERCY LAB CV	
Calcium	8.9	8.4 - 10.2 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Phosphorus	5.6	2.5 - 4.5 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Albumin	3.4	3.5 - 5.0 g/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/14/2024 5:47 AM PST	02/14/2024 6:00 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/13/2024 7:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
RBC	3.20	3.98 - 5.25 M/mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
HGB	9.8	11.9 - 15.3 g/dL		02/13/2024 8:01 AM PST	MERCY LAB CV	
HCT	32.2	37.3 - 46.7 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/13/2024 8:01 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/13/2024 8:01 AM PST	MERCY LAB CV	
MCHC	30.4	31.0 - 36.0 g/dL		02/13/2024 8:01 AM PST	MERCY LAB CV	
RDW	13.8	11.7 - 14.9 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Platelet Count	333	150 - 425 K/mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
MPV	8.3	9.0 - 12.8 fL		02/13/2024 8:01 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
nRBC	0.4	0.0 /100 WBCs		02/13/2024 8:01 AM PST	MERCY LAB CV	
Neutrophils %	44.1	38.0 - 74.0 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
IG%	0.4	0.0 - 1.2 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Lymphocytes %	40.9	16.0 - 48.0 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Monocytes %	8.9	4.9 - 12.5 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Eosinophils %	4.9	0.0 - 9.5 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.35	1.50 - 7.40 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.17	0.90 - 3.10 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Monocytes	0.47	0.26 - 0.87 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.26	0.00 - 0.51 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.35	1.50 - 7.40 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/13/2024 7:36 AM PST	02/13/2024 7:40 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/13/2024 7:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	95	70 - 125 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Sodium	139	137 - 145 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Urea Nitrogen	22	7 - 17 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		02/13/2024 8:03 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Osmolality Calculated	291	280 - 305 mOsm/kg H20		02/13/2024 8:03 AM PST	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Phosphorus	5.1	2.5 - 4.5 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/13/2024 7:36 AM PST	02/13/2024 7:40 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/13/2024 7:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/13/2024 8:03 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	95	70 - 125 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Sodium	139	137 - 145 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Urea Nitrogen	22	7 - 17 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		02/13/2024 8:03 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Osmolality Calculated	291	280 - 305 mOsm/kg H20		02/13/2024 8:03 AM PST	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Protein, total	8.4	6.3 - 8.2 g/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Alkaline Phosphatase	79	38 - 126 Units/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
AST	35	14 - 36 Units/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
ALT	25	<=34 Units/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/13/2024 7:36 AM PST	02/13/2024 7:40 AM PST

Narrative

MERCY LAB CV - 02/13/2024 8:03 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/12/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	97	70 - 125 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Urea Nitrogen	21	7 - 17 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Creatinine	0.81	0.50 - 1.00 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	84	>60 mL/min/1.73m*2		02/12/2024 6:37 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Osmolality Calculated	289	280 - 305 mOsm/kg H20		02/12/2024 6:37 AM PST	MERCY LAB CV	
Calcium	9.0	8.4 - 10.2 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Phosphorus	5.3	2.5 - 4.5 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Albumin	3.5	3.5 - 5.0 g/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/12/2024 5:28 AM PST	02/12/2024 6:12 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/11/2024 8:09 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	113	70 - 125 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Urea Nitrogen	23	7 - 17 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Creatinine	0.84	0.50 - 1.00 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	80	>60 mL/min/1.73m*2		02/11/2024 8:46 PM PST	MERCY LAB CV	
Anion Gap	4	6 - 14 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Osmolality Calculated	290	280 - 305 mOsm/kg H20		02/11/2024 8:46 PM PST	MERCY LAB CV	
Calcium	8.8	8.4 - 10.2 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Phosphorus	4.3	2.5 - 4.5 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Albumin	3.6	3.5 - 5.0 g/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/11/2024 8:09 PM PST	02/11/2024 8:20 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray hip with and without pelvis right 2-3 views (X-RAY HIP RIGHT 2-3 VIEWS) - Final result (02/11/2024 4:26 PM PST)

Anatomical Region	Laterality	Modality
Hip	Right	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/11/2024 4:42 PM PST	

Impressions

02/12/2024 11:04 AM PST
IMPRESSION:
1. Mild degenerative changes at right hip joint. No acute fracture detected.

release

Narrative

02/12/2024 11:04 AM PST
EXAMINATION: X-RAY HIP RIGHT 2-3 VIEWS

DATE/TIME: 2/11/2024 4:26 PM

HISTORY: Right hip pain

COMPARISON: None.

FINDINGS:

Two upright views of the right hip obtained.

There is normal alignment of the right hip joint without dislocation. There is mild superior joint space narrowing at the right hip joint. Minimal degenerative spurring seen. No plain film evident acute fracture at the right hip detected.

There is normal contour and density of the right femoral head.

Mild degenerative changes of the pubic symphysis seen.

Procedure Note

Jerome Pierce, MD - 02/12/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY HIP RIGHT 2-3 VIEWS

DATE/TIME: 2/11/2024 4:26 PM

HISTORY: Right hip pain

COMPARISON: None.

FINDINGS:

Two upright views of the right hip obtained.

There is normal alignment of the right hip joint without dislocation. There is mild superior joint space narrowing at the right hip joint. Minimal degenerative spurring seen. No plain film evident acute fracture at the right hip detected.

There is normal contour and density of the right femoral head.

Mild degenerative changes of the pubic symphysis seen.

IMPRESSION:
IMPRESSION:
1. Mild degenerative changes at right hip joint. No acute fracture detected.

release

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (02/11/2024 4:08 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/11/2024 4:11 PM PST	

Impressions

02/11/2024 4:11 PM PST
IMPRESSION: No deep venous thrombosis detected in either leg.

release

Narrative

02/11/2024 4:11 PM PST

Narrative

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 2/11/2024 4:08 PM

HISTORY: Dvt

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable. The technologist could not see the left peroneal vein possibly due to presence of edema.
OTHER: None.

Procedure Note

Jerome Pierce, MD - 02/11/2024

Formatting of this note might be different from the original.

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 2/11/2024 4:08 PM

HISTORY: Dvt

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable. The technologist could not see the left peroneal vein possibly due to presence of edema.
OTHER: None.

IMPRESSION:
IMPRESSION: No deep venous thrombosis detected in either leg.

release

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	IMG US PROCEDURES	Final Result

12-LEAD ECG - Final result (02/11/2024 10:50 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	98	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	98	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	184	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	90	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	386	ms			SCRIPPS RADIOLOGY	
QTC	492	ms			SCRIPPS RADIOLOGY	
P AXIS	25	degrees			SCRIPPS RADIOLOGY	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
R AXIS	55	degrees			SCRIPPS RADIOLOGY	
T AXIS	133	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/11/2024 10:50 AM PST	02/23/2024 3:34 PM PST

Impressions

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SCRIPPS RADIOLOGY - 02/23/2024 3:34 PM PST
Normal sinus rhythm
ST & T wave abnormality, consider lateral ischemia
Abnormal ECG
```

Narrative

This result has an attachment that is not available.

Procedure Note

Raghava Gollapudi, MD - 02/23/2024

Formatting of this note might be different from the original.

IMPRESSION:
Normal sinus rhythm
ST & T wave abnormality, consider lateral ischemia
Abnormal ECG

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

CBC WITH DIFFERENTIAL - Final result (02/10/2024 5:52 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	6.0	3.4 - 11.0 K/mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
RBC	3.25	3.98 - 5.25 M/mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
HGB	10.1	11.9 - 15.3 g/dL		02/10/2024 6:39 AM PST	MERCY LAB CV	
HCT	32.4	37.3 - 46.7 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/10/2024 6:39 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/10/2024 6:39 AM PST	MERCY LAB CV	
MCHC	31.2	31.0 - 36.0 g/dL		02/10/2024 6:39 AM PST	MERCY LAB CV	
RDW	13.8	11.7 - 14.9 %		02/10/2024 6:39 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	377	150 - 425 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
MPV	8.4	9.0 - 12.8 fL		02/10/2024 6:39 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/10/2024 6:39 AM PST	MERCY LAB CV	
Neutrophils %	45.9	38.0 - 74.0 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
IG%	0.3	0.0 - 1.2 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Lymphocytes %	41.3	16.0 - 48.0 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Monocytes %	8.2	4.9 - 12.5 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Eosinophils %	3.5	0.0 - 9.5 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.74	1.50 - 7.40 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.47	0.90 - 3.10 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Monocytes	0.49	0.26 - 0.87 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.21	0.00 - 0.51 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.74	1.50 - 7.40 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/10/2024 5:52 AM PST	02/10/2024 6:32 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/10/2024 5:52 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/10/2024 7:00 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Urea Nitrogen	21	7 - 17 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/10/2024 7:00 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Osmolality Calculated	277	280 - 305 mOsm/kg H20		02/10/2024 7:00 AM PST	MERCY LAB CV	
Calcium	9.0	8.4 - 10.2 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Protein, total	8.5	6.3 - 8.2 g/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Albumin	3.6	3.5 - 5.0 g/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Alkaline Phosphatase	80	38 - 126 Units/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
AST	33	14 - 36 Units/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
ALT	21	<=34 Units/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/10/2024 5:52 AM PST	02/10/2024 6:31 AM PST

Narrative

MERCY LAB CV - 02/10/2024 7:00 AM PST

```
for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (02/09/2024 10:58 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/09/2024 11:35 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	
Sodium	136	137 - 145 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Urea Nitrogen	22	7 - 17 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	
Creatinine	1.00	0.50 - 1.00 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	65	>60 mL/min/1.73m*2		02/09/2024 11:35 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Osmolality Calculated	285	280 - 305 mOsm/kg H20		02/09/2024 11:35 AM PST	MERCY LAB CV	
Calcium	8.9	8.4 - 10.2 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/09/2024 10:58 AM PST	02/09/2024 11:18 AM PST

Narrative

MERCY LAB CV - 02/09/2024 11:35 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray ankle left 3 or more views (X-RAY ANKLE LEFT 3 OR MORE VIEWS) - Final result (02/07/2024 5:27 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities, Ankle	Left	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/07/2024 5:56 PM PST	

Impressions

02/07/2024 5:57 PM PST
IMPRESSION:
1. Severe soft tissue swelling around the ankle midfoot and forefoot correlate clinically for cellulitis. No radiographic evidence of osteomyelitis. Midfoot degenerative changes

2. No soft tissue emphysema

release

Narrative

02/07/2024 5:57 PM PST
EXAMINATION: X-RAY ANKLE LEFT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle pain, no prior imaging

COMPARISON: None.

FINDINGS: See impression.

Procedure Note

George Wesbey, MD - 02/07/2024

Procedure Note

Formatting of this note might be different from the original.
EXAMINATION: X-RAY ANKLE LEFT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle pain, no prior imaging

COMPARISON: None.

FINDINGS: See impression.

IMPRESSION:
IMPRESSION:
1. Severe soft tissue swelling around the ankle midfoot and forefoot correlate clinically for cellulitis. No radiographic evidence of osteomyelitis. Midfoot degenerative changes

2. No soft tissue emphysema

release

Authorizing Provider	Result Type	Result Status
Howie Que DPM	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

X-ray ankle right 3 or more views (X-RAY ANKLE RIGHT 3 OR MORE VIEWS) - Final result (02/07/2024 5:27 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities, Ankle	Right	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/07/2024 5:57 PM PST	

Impressions

02/07/2024 5:58 PM PST
IMPRESSION:
1. Marked soft tissue swelling at the ankle and hindfoot and midfoot and forefoot

2. Small plantar calcaneal spur

3. No fractures or dislocations identified

release

Narrative

02/07/2024 5:58 PM PST
EXAMINATION: X-RAY ANKLE RIGHT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle trauma, fracture, xray done (Age >= 5y)

COMPARISON: None.

FINDINGS: See impression.

Procedure Note

George Wesbey, MD - 02/07/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY ANKLE RIGHT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle trauma, fracture, xray done (Age >= 5y)

COMPARISON: None.

FINDINGS: See impression.

IMPRESSION:
IMPRESSION:
1. Marked soft tissue swelling at the ankle and hindfoot and midfoot and forefoot

2. Small plantar calcaneal spur

3. No fractures or dislocations identified

release

Authorizing Provider	Result Type	Result Status
Howie Que DPM	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (02/07/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
RBC	2.96	3.98 - 5.25 M/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
HGB	9.2	11.9 - 15.3 g/dL		02/07/2024 6:51 AM PST	MERCY LAB CV	
HCT	29.5	37.3 - 46.7 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/07/2024 6:51 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/07/2024 6:51 AM PST	MERCY LAB CV	
MCHC	31.2	31.0 - 36.0 g/dL		02/07/2024 6:51 AM PST	MERCY LAB CV	
RDW	14.1	11.7 - 14.9 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Platelet Count	446	150 - 425 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
MPV	8.3	9.0 - 12.8 fL		02/07/2024 6:51 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/07/2024 6:51 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Neutrophils %	44.2	38.0 - 74.0 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Lymphocytes %	41.7	16.0 - 48.0 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Monocytes %	9.2	4.9 - 12.5 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Eosinophils %	3.2	0.0 - 9.5 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Basophils %	0.9	0.0 - 1.6 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.34	1.50 - 7.40 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.21	0.90 - 3.10 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Monocytes	0.49	0.26 - 0.87 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.17	0.00 - 0.51 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.34	1.50 - 7.40 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/07/2024 5:35 AM PST	02/07/2024 6:34 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/07/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/07/2024 7:34 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	94	70 - 125 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/07/2024 7:34 AM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Osmolality Calculated	274	280 - 305 mOsm/kg H20		02/07/2024 7:34 AM PST	MERCY LAB CV	
Calcium	8.8	8.4 - 10.2 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Protein, total	8.0	6.3 - 8.2 g/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Albumin	3.3	3.5 - 5.0 g/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Alkaline Phosphatase	90	38 - 126 Units/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
AST	32	14 - 36 Units/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
ALT	21	<=34 Units/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/07/2024 5:35 AM PST	02/07/2024 6:34 AM PST

Narrative

MERCY LAB CV - 02/07/2024 7:34 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/06/2024 5:29 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.0	3.4 - 11.0 K/mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
RBC	3.06	3.98 - 5.25 M/mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
HGB	9.1	11.9 - 15.3 g/dL		02/06/2024 6:32 AM PST	MERCY LAB CV	
HCT	30.5	37.3 - 46.7 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/06/2024 6:32 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		02/06/2024 6:32 AM PST	MERCY LAB CV	
MCHC	29.8	31.0 - 36.0 g/dL		02/06/2024 6:32 AM PST	MERCY LAB CV	
RDW	14.2	11.7 - 14.9 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Platelet Count	472	150 - 425 K/mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
MPV	8.2	9.0 - 12.8 fL		02/06/2024 6:32 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/06/2024 6:32 AM PST	MERCY LAB CV	
Neutrophils %	42.6	38.0 - 74.0 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
IG%	0.6	0.0 - 1.2 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Lymphocytes %	42.5	16.0 - 48.0 %		02/06/2024 6:32 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	10.5	4.9 - 12.5 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Eosinophils %	2.8	0.0 - 9.5 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.11	1.50 - 7.40 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.11	0.90 - 3.10 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Monocytes	0.52	0.26 - 0.87 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.14	0.00 - 0.51 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.11	1.50 - 7.40 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/06/2024 5:29 AM PST	02/06/2024 6:04 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/06/2024 5:29 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/06/2024 6:36 AM PST	MERCY LAB CV	
Glucose	103	70 - 125 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Potassium	4.1	3.5 - 5.1 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	103	98 - 107 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
CO2, Total	25	22 - 30 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Creatinine	0.71	0.50 - 1.00 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	98	>60 mL/min/1.73m*2		02/06/2024 6:36 AM PST	MERCY LAB CV	
Anion Gap	4	6 - 14 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Osmolality Calculated	275	280 - 305 mOsm/kg H20		02/06/2024 6:36 AM PST	MERCY LAB CV	
Calcium	8.8	8.4 - 10.2 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Protein, total	7.9	6.3 - 8.2 g/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Albumin	3.3	3.5 - 5.0 g/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Alkaline Phosphatase	84	38 - 126 Units/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
AST	36	14 - 36 Units/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
ALT	26	<=34 Units/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/06/2024 5:29 AM PST	02/06/2024 6:03 AM PST

Narrative

MERCY LAB CV - 02/06/2024 6:36 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/05/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
RBC	2.88	3.98 - 5.25 M/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
HGB	8.8	11.9 - 15.3 g/dL		02/05/2024 7:25 AM PST	MERCY LAB CV	
HCT	29.0	37.3 - 46.7 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/05/2024 7:25 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/05/2024 7:25 AM PST	MERCY LAB CV	
MCHC	30.3	31.0 - 36.0 g/dL		02/05/2024 7:25 AM PST	MERCY LAB CV	
RDW	13.9	11.7 - 14.9 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Platelet Count	515	150 - 425 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
MPV	8.1	9.0 - 12.8 fL		02/05/2024 7:25 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/05/2024 7:25 AM PST	MERCY LAB CV	
Neutrophils %	49.7	38.0 - 74.0 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Lymphocytes %	35.5	16.0 - 48.0 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Monocytes %	10.2	4.9 - 12.5 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Eosinophils %	3.0	0.0 - 9.5 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.64	1.50 - 7.40 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	1.88	0.90 - 3.10 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Monocytes	0.54	0.26 - 0.87 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.16	0.00 - 0.51 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.64	1.50 - 7.40 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/05/2024 5:53 AM PST	02/05/2024 6:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/05/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/05/2024 7:10 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Chloride	98	98 - 107 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Urea Nitrogen	15	7 - 17 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Creatinine	0.75	0.50 - 1.00 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	92	>60 mL/min/1.73m*2		02/05/2024 7:10 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Anion Gap	4	6 - 14 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Osmolality Calculated	273	280 - 305 mOsm/kg H20		02/05/2024 7:10 AM PST	MERCY LAB CV	
Calcium	8.7	8.4 - 10.2 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Protein, total	8.0	6.3 - 8.2 g/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Albumin	3.2	3.5 - 5.0 g/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Alkaline Phosphatase	107	38 - 126 Units/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
AST	39	14 - 36 Units/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
ALT	22	<=34 Units/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/05/2024 5:53 AM PST	02/05/2024 6:43 AM PST

Narrative

MERCY LAB CV - 02/05/2024 7:10 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/04/2024 4:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.1	3.4 - 11.0 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
RBC	2.67	3.98 - 5.25 M/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HGB	8.0	11.9 - 15.3 g/dL		02/04/2024 5:35 AM PST	MERCY LAB CV	
HCT	27.2	37.3 - 46.7 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		02/04/2024 5:35 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		02/04/2024 5:35 AM PST	MERCY LAB CV	
MCHC	29.4	31.0 - 36.0 g/dL		02/04/2024 5:35 AM PST	MERCY LAB CV	
RDW	13.9	11.7 - 14.9 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Platelet Count	461	150 - 425 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		02/04/2024 5:35 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/04/2024 5:35 AM PST	MERCY LAB CV	
Neutrophils %	44.3	38.0 - 74.0 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
IG%	1.2	0.0 - 1.2 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Lymphocytes %	38.5	16.0 - 48.0 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Monocytes %	10.8	4.9 - 12.5 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Eosinophils %	4.2	0.0 - 9.5 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Neutrophils	1.81	1.50 - 7.40 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
IG#	0.05	0.00 - 0.10 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.57	0.90 - 3.10 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Monocytes	0.44	0.26 - 0.87 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.17	0.00 - 0.51 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Preliminary Abs Neut	1.81	1.50 - 7.40 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/04/2024 4:11 AM PST	02/04/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/04/2024 4:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/04/2024 6:47 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Potassium	3.9	3.5 - 5.1 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/04/2024 6:47 AM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Osmolality Calculated	273	280 - 305 mOsm/kg H20		02/04/2024 6:47 AM PST	MERCY LAB CV	
Calcium	8.7	8.4 - 10.2 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protein, total	7.3	6.3 - 8.2 g/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Albumin	2.9	3.5 - 5.0 g/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Alkaline Phosphatase	81	38 - 126 Units/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
AST	38	14 - 36 Units/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
ALT	22	<=34 Units/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Bilirubin, Total	<0.1	0.2 - 1.3 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/04/2024 4:11 AM PST	02/04/2024 5:24 AM PST

Narrative

MERCY LAB CV - 02/04/2024 6:47 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/03/2024 5:42 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.5	3.4 - 11.0 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
RBC	2.62	3.98 - 5.25 M/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
HGB	8.0	11.9 - 15.3 g/dL		02/03/2024 6:07 AM PST	MERCY LAB CV	
HCT	26.4	37.3 - 46.7 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/03/2024 6:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCH	31	26 - 33 pg		02/03/2024 6:07 AM PST	MERCY LAB CV	
MCHC	30.3	31.0 - 36.0 g/dL		02/03/2024 6:07 AM PST	MERCY LAB CV	
RDW	14.1	11.7 - 14.9 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Platelet Count	496	150 - 425 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
MPV	7.7	9.0 - 12.8 fL		02/03/2024 6:07 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/03/2024 6:07 AM PST	MERCY LAB CV	
Neutrophils %	52.8	38.0 - 74.0 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
IG%	1.3	0.0 - 1.2 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Lymphocytes %	32.7	16.0 - 48.0 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Monocytes %	8.5	4.9 - 12.5 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Eosinophils %	4.0	0.0 - 9.5 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.36	1.50 - 7.40 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
IG#	0.06	0.00 - 0.10 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.46	0.90 - 3.10 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.38	0.26 - 0.87 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.18	0.00 - 0.51 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.36	1.50 - 7.40 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/03/2024 5:42 AM PST	02/03/2024 5:48 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/03/2024 5:42 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/03/2024 6:56 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Urea Nitrogen	19	7 - 17 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Creatinine	0.82	0.50 - 1.00 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	82	>60 mL/min/1.73m*2		02/03/2024 6:56 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Osmolality Calculated	274	280 - 305 mOsm/kg H20		02/03/2024 6:56 AM PST	MERCY LAB CV	
Calcium	8.6	8.4 - 10.2 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Protein, total	7.5	6.3 - 8.2 g/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Albumin	3.0	3.5 - 5.0 g/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Alkaline Phosphatase	76	38 - 126 Units/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
AST	42	14 - 36 Units/L		02/03/2024 6:56 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ALT	24	<=34 Units/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/03/2024 5:42 AM PST	02/03/2024 5:48 AM PST

Narrative

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MERCY LAB CV - 02/03/2024 6:56 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/02/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
RBC	2.59	3.98 - 5.25 M/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
HGB	7.9	11.9 - 15.3 g/dL		02/02/2024 5:47 AM PST	MERCY LAB CV	
HCT	26.2	37.3 - 46.7 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/02/2024 5:47 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/02/2024 5:47 AM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		02/02/2024 5:47 AM PST	MERCY LAB CV	
RDW	13.9	11.7 - 14.9 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Platelet Count	513	150 - 425 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MPV	7.8	9.0 - 12.8 fL		02/02/2024 5:47 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/02/2024 5:47 AM PST	MERCY LAB CV	
Neutrophils %	44.9	38.0 - 74.0 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
IG%	1.1	0.0 - 1.2 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Lymphocytes %	39.9	16.0 - 48.0 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Monocytes %	8.6	4.9 - 12.5 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Eosinophils %	4.4	0.0 - 9.5 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Basophils %	1.1	0.0 - 1.6 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.36	1.50 - 7.40 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
IG#	0.06	0.00 - 0.10 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.10	0.90 - 3.10 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Monocytes	0.45	0.26 - 0.87 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.23	0.00 - 0.51 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Basophils	0.06	0.00 - 0.09 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.36	1.50 - 7.40 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/02/2024 5:21 AM PST	02/02/2024 5:34 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/02/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/02/2024 5:50 AM PST	MERCY LAB CV	
Glucose	88	70 - 125 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Sodium	130	137 - 145 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Urea Nitrogen	19	7 - 17 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/02/2024 5:50 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Osmolality Calculated	272	280 - 305 mOsm/kg H20		02/02/2024 5:50 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Protein, total	7.2	6.3 - 8.2 g/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Alkaline Phosphatase	71	38 - 126 Units/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
AST	28	14 - 36 Units/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Bilirubin, Total	<0.1	0.2 - 1.3 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/02/2024 5:21 AM PST	02/02/2024 5:33 AM PST

Narrative

MERCY LAB CV - 02/02/2024 5:50 AM PST

Narrative

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for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/01/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.2	3.4 - 11.0 K/mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
RBC	2.45	3.98 - 5.25 M/mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
HGB	7.5	11.9 - 15.3 g/dL		02/01/2024 6:49 AM PST	MERCY LAB CV	
HCT	24.6	37.3 - 46.7 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/01/2024 6:49 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/01/2024 6:49 AM PST	MERCY LAB CV	
MCHC	30.5	31.0 - 36.0 g/dL		02/01/2024 6:49 AM PST	MERCY LAB CV	
RDW	13.5	11.7 - 14.9 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Platelet Count	573	150 - 425 K/mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		02/01/2024 6:49 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/01/2024 6:49 AM PST	MERCY LAB CV	
Neutrophils %	44.4	38.0 - 74.0 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
IG%	1.0	0.0 - 1.2 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Lymphocytes %	39.6	16.0 - 48.0 %		02/01/2024 6:49 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	9.8	4.9 - 12.5 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Eosinophils %	4.2	0.0 - 9.5 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.33	1.50 - 7.40 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
IG#	0.05	0.00 - 0.10 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.07	0.90 - 3.10 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Monocytes	0.51	0.26 - 0.87 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.22	0.00 - 0.51 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.33	1.50 - 7.40 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/01/2024 5:45 AM PST	02/01/2024 6:32 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/01/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/01/2024 7:09 AM PST	MERCY LAB CV	
Glucose	90	70 - 125 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Sodium	135	137 - 145 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	102	98 - 107 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Urea Nitrogen	17	7 - 17 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		02/01/2024 7:09 AM PST	MERCY LAB CV	
Anion Gap	-3	6 - 14 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Osmolality Calculated	281	280 - 305 mOsm/kg H20		02/01/2024 7:09 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Protein, total	6.8	6.3 - 8.2 g/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Alkaline Phosphatase	71	38 - 126 Units/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
AST	28	14 - 36 Units/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Bilirubin, Total	<0.1	0.2 - 1.3 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/01/2024 5:45 AM PST	02/01/2024 6:32 AM PST

Narrative

MERCY LAB CV - 02/01/2024 7:09 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/31/2024 6:10 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/31/2024 7:09 AM PST	MERCY LAB CV	
Glucose	92	70 - 125 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Sodium	135	137 - 145 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/31/2024 7:09 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Osmolality Calculated	282	280 - 305 mOsm/kg H20		01/31/2024 7:09 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Protein, total	7.3	6.3 - 8.2 g/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Albumin	3.0	3.5 - 5.0 g/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Alkaline Phosphatase	88	38 - 126 Units/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
AST	32	14 - 36 Units/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
ALT	17	<=34 Units/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:10 AM PST	01/31/2024 6:38 AM PST

Narrative

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MERCY LAB CV - 01/31/2024 7:09 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.6	3.4 - 11.0 K/mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
RBC	2.57	3.98 - 5.25 M/mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
HGB	8.0	11.9 - 15.3 g/dL		01/31/2024 7:35 AM PST	MERCY LAB CV	
HCT	26.1	37.3 - 46.7 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/31/2024 7:35 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/31/2024 7:35 AM PST	MERCY LAB CV	
MCHC	30.7	31.0 - 36.0 g/dL		01/31/2024 7:35 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Platelet Count	598	150 - 425 K/mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		01/31/2024 7:35 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/31/2024 7:35 AM PST	MERCY LAB CV	
Neutrophils %	45.8	38.0 - 74.0 %		01/31/2024 7:35 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
IG%	0.5	0.0 - 1.2 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Lymphocytes %	39.7	16.0 - 48.0 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Monocytes %	7.9	4.9 - 12.5 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Eosinophils %	5.0	0.0 - 9.5 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Basophils %	1.1	0.0 - 1.6 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.56	1.50 - 7.40 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.22	0.90 - 3.10 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Monocytes	0.44	0.26 - 0.87 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.28	0.00 - 0.51 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Basophils	0.06	0.00 - 0.09 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.56	1.50 - 7.40 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:38 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

IRON +TIBC (IRON AND TIBC) - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Iron	34	50 - 170 mcg/dL		01/31/2024 12:29 PM PST	SCRIPPS LAB	
Transferrin	128	180 - 382 mg/ dL		01/31/2024 12:29 PM PST	SCRIPPS LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Iron Binding Capacity Calculated	160	250 - 450 mcg/dL		01/31/2024 12:29 PM PST	SCRIPPS LAB	
Iron Saturation	21	20 - 50 %		01/31/2024 12:29 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:38 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

FOLATE - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Folate	10.5	7.0 - 31.4 ng/mL		01/31/2024 12:58 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:39 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

VITAMIN B12 - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Vitamin B-12	718	213 - 816 pg/mL		01/31/2024 12:58 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:39 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

IR PICC LINE PLACEMENT - US/FLUROR GUIDANCE AS NEEDED (IR PICC LINE PLACEMENT - US/FLUORO GUIDANCE AS NEEDED) - Final result (01/30/2024 4:05 PM PST)

Anatomical Region	Laterality	Modality
		IR/OR Hybrid Labs

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/30/2024 4:11 PM PST	

Impressions

01/30/2024 4:11 PM PST
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance.

release

Narrative

01/30/2024 4:11 PM PST
EXAMINATION: IR PICC LINE PLACEMENT - US/FLUORO GUIDANCE AS NEEDED

DATE/TIME: 1/30/2024 4:05 PM

HISTORY: for long-term antibiotic administration

COMPARISON: None.

TECHNIQUE: A description of the procedure, including risks, complications and benefits of the procedure were explained to the patient who gave written and oral consent. The skin around the access site was prepped using maximal sterile barrier preparation technique which included caps, masks, sterile gowns, and sterile gloves worn by all personnel. In addition, the following was also utilized: hand hygiene, a large sterile sheet/drape, and 2% chlorhexidine for cutaneous antisepsis. In addition, sterile gel and a sterile transducer probe cover was utilized. Real-time ultrasound was used to evaluate potential access sites for venous access, and was used to directly visualize needle entry into the selected vein lumen. Using local anesthetic, the access vein was punctured with a 21G needle and an 0.018 inch guidewire was placed. A 5F sheath was placed and the PICC line was advanced into position using fluoroscopic guidance. The line was secured to the skin and a sterile dressing was
applied.

FLUOROSCOPY: Time 0.1-minute, One fluoroscopic spot image(s) obtained

FINDINGS:
VEIN ACCESSED: Right basilic vein.
CATHETER: 4 French Power PICC
CATHETER TIP POSITION: Distal superior vena cava.
MEDICATIONS: Local anesthetic.
IMAGING FINDINGS: No sonographic or fluoroscopic abnormalities.
OTHER: None.

Procedure Note

Trevor Nelson, MD - 01/30/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: IR PICC LINE PLACEMENT - US/FLUORO GUIDANCE AS NEEDED

DATE/TIME: 1/30/2024 4:05 PM

HISTORY: for long-term antibiotic administration

COMPARISON: None.

TECHNIQUE: A description of the procedure, including risks, complications and benefits of the procedure were explained to the patient who gave written and oral consent. The skin around the access site was prepped using maximal sterile barrier preparation technique which included caps, masks, sterile gowns, and sterile gloves worn by all personnel. In addition, the following was also utilized: hand hygiene, a large sterile sheet/drape, and 2% chlorhexidine for cutaneous antisepsis. In addition, sterile gel and a sterile transducer probe cover was utilized. Real-time ultrasound was used to evaluate potential access sites for venous access, and was used to directly visualize needle entry into the selected vein lumen. Using local anesthetic, the access vein was punctured with a 21G needle and an 0.018 inch guidewire was placed. A 5F sheath was placed and the PICC line was advanced into position using fluoroscopic guidance. The line was secured to the skin and a sterile dressing was
applied.

FLUOROSCOPY: Time 0.1-minute, One fluoroscopic spot image(s) obtained

FINDINGS:
VEIN ACCESSED: Right basilic vein.
CATHETER: 4 French Power PICC
CATHETER TIP POSITION: Distal superior vena cava.
MEDICATIONS: Local anesthetic.
IMAGING FINDINGS: No sonographic or fluoroscopic abnormalities.
OTHER: None.

IMPRESSION:
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance.

release

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	IMG IR PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/30/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.3	3.4 - 11.0 K/mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
RBC	2.33	3.98 - 5.25 M/mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
HGB	7.1	11.9 - 15.3 g/dL		01/30/2024 5:50 AM PST	MERCY LAB CV	
HCT	23.8	37.3 - 46.7 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/30/2024 5:50 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/30/2024 5:50 AM PST	MERCY LAB CV	
MCHC	29.8	31.0 - 36.0 g/dL		01/30/2024 5:50 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
RDW	13.4	11.7 - 14.9 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Platelet Count	496	150 - 425 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		01/30/2024 5:50 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/30/2024 5:50 AM PST	MERCY LAB CV	
Neutrophils %	41.3	38.0 - 74.0 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
IG%	0.5	0.0 - 1.2 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Lymphocytes %	38.3	16.0 - 48.0 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Monocytes %	11.7	4.9 - 12.5 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Eosinophils %	7.0	0.0 - 9.5 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Basophils %	1.2	0.0 - 1.6 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Neutrophils	1.77	1.50 - 7.40 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.64	0.90 - 3.10 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Monocytes	0.50	0.26 - 0.87 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.30	0.00 - 0.51 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Preliminary Abs Neut	1.77	1.50 - 7.40 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/30/2024 5:21 AM PST	01/30/2024 5:43 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/30/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/30/2024 6:00 AM PST	MERCY LAB CV	
Glucose	100	70 - 125 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Sodium	135	137 - 145 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Urea Nitrogen	27	7 - 17 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Creatinine	1.22	0.50 - 1.00 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	51	>60 mL/min/1.73m*2		01/30/2024 6:00 AM PST	MERCY LAB CV	
Anion Gap	-1	6 - 14 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Osmolality Calculated	285	280 - 305 mOsm/kg H20		01/30/2024 6:00 AM PST	MERCY LAB CV	
Calcium	8.1	8.4 - 10.2 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Protein, total	6.4	6.3 - 8.2 g/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Alkaline Phosphatase	75	38 - 126 Units/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
AST	23	14 - 36 Units/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
ALT	14	<=34 Units/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/30/2024 5:21 AM PST	01/30/2024 5:40 AM PST

Narrative

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MERCY LAB CV - 01/30/2024 6:00 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (01/29/2024 4:18 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	>140	<=30 mm/hr		01/29/2024 5:42 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 4:18 PM PST	01/29/2024 4:38 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (01/29/2024 4:18 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	20.6	<10.0 mg/L		01/29/2024 4:58 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 4:18 PM PST	01/29/2024 4:38 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CT cervical spine with contrast (CT CERVICAL SPINE WITH CONTRAST) - Final result (01/29/2024 3:23 PM PST)

Anatomical Region	Laterality	Modality
C-spine		Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/29/2024 3:27 PM PST	

Impressions

01/29/2024 3:35 PM PST
IMPRESSION:

1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis.
2. Persistent prevertebral fluid, probably similar in extent to the recent MRI.

release

Narrative

01/29/2024 3:35 PM PST
EXAMINATION: CT CERVICAL SPINE WITH CONTRAST

DATE/TIME: 1/29/2024 3:23 PM

HISTORY: Prevertebral, retropharyngeal fluid collection

COMPARISON: MRI cervical spine 1/26/2024

TECHNIQUE: CT examination was performed with intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

There is reversal of cervical lordosis. There is C2-3 and C7-T1 mild anterolisthesis.

There are erosive changes of the C4-5 endplates. There is also irregularity of the C5-6 endplates.

There is persistent prevertebral fluid, extending from C2-C7.

There are multilevel degenerative changes, most pronounced at C6-7 where a disc osteophyte complex and uncovertebral hypertrophy cause mild canal stenosis and moderate left and mild right foraminal stenosis.

There is a partially visualized right pleural effusion.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 19; DLP: 317

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

Procedure Note

Benjamin Freeze, MD - 01/29/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: CT CERVICAL SPINE WITH CONTRAST

DATE/TIME: 1/29/2024 3:23 PM

HISTORY: Prevertebral, retropharyngeal fluid collection

COMPARISON: MRI cervical spine 1/26/2024

TECHNIQUE: CT examination was performed with intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

There is reversal of cervical lordosis. There is C2-3 and C7-T1 mild anterolisthesis.

There are erosive changes of the C4-5 endplates. There is also irregularity of the C5-6 endplates.

There is persistent prevertebral fluid, extending from C2-C7.

There are multilevel degenerative changes, most pronounced at C6-7 where a disc osteophyte complex and uncovertebral hypertrophy cause mild canal stenosis and moderate left and mild right foraminal stenosis.

There is a partially visualized right pleural effusion.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 19; DLP: 317

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

IMPRESSION:
IMPRESSION:

1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis.
2. Persistent prevertebral fluid, probably similar in extent to the recent MRI.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG CT PROCEDURES	Final Result

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (01/29/2024 2:59 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/29/2024 3:19 PM PST	

Impressions

01/29/2024 3:20 PM PST

Impressions

IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Narrative

01/29/2024 3:20 PM PST

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/29/2024 2:59 PM

HISTORY: bilateral leg edema

COMPARISON: 1/16/2024

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

Procedure Note

Paul Rickards, MD - 01/29/2024

Formatting of this note might be different from the original.

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/29/2024 2:59 PM

HISTORY: bilateral leg edema

COMPARISON: 1/16/2024

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

IMPRESSION:
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG US PROCEDURES	Final Result

X-ray chest 2 views with pa/ap and lateral (X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL) - Final result (01/29/2024 9:22 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/29/2024 9:26 AM PST	

Impressions

01/29/2024 9:26 AM PST
IMPRESSION: No significant change from prior.

release

Narrative

01/29/2024 9:26 AM PST
EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 1/29/2024 9:22 AM

HISTORY: f/u pleural fluid collection

COMPARISON: 1/25/2024

FINDINGS:

1. Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.
2. Clear left lung.
3. Stable cardiac silhouette size.

Procedure Note

Harold Pimenta, MD - 01/29/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 1/29/2024 9:22 AM

HISTORY: f/u pleural fluid collection

COMPARISON: 1/25/2024

FINDINGS:

1. Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.
2. Clear left lung.
3. Stable cardiac silhouette size.

IMPRESSION:
IMPRESSION: No significant change from prior.

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.6	3.4 - 11.0 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
RBC	2.48	3.98 - 5.25 M/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HGB	7.7	11.9 - 15.3 g/dL		01/29/2024 7:23 AM PST	MERCY LAB CV	
HCT	25.2	37.3 - 46.7 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/29/2024 7:23 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/29/2024 7:23 AM PST	MERCY LAB CV	
MCHC	30.6	31.0 - 36.0 g/dL		01/29/2024 7:23 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Platelet Count	552	150 - 425 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
MPV	8.4	9.0 - 12.8 fL		01/29/2024 7:23 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/29/2024 7:23 AM PST	MERCY LAB CV	
Neutrophils %	53.3	38.0 - 74.0 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
IG%	0.6	0.0 - 1.2 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Lymphocytes %	27.5	16.0 - 48.0 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Monocytes %	10.2	4.9 - 12.5 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Eosinophils %	7.1	0.0 - 9.5 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Basophils %	1.3	0.0 - 1.6 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.46	1.50 - 7.40 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.27	0.90 - 3.10 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Monocytes	0.47	0.26 - 0.87 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.33	0.00 - 0.51 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Basophils	0.06	0.00 - 0.09 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.46	1.50 - 7.40 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 7:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	>140	<=30 mm/hr		01/29/2024 11:20 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 7:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	26.8	<10.0 mg/L		01/29/2024 10:22 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 6:07 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/29/2024 6:52 AM PST	MERCY LAB CV	
Glucose	101	70 - 125 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Chloride	97	98 - 107 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Creatinine	0.75	0.50 - 1.00 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	92	>60 mL/min/1.73m*2		01/29/2024 6:52 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Osmolality Calculated	275	280 - 305 mOsm/kg H20		01/29/2024 6:52 AM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Protein, total	7.1	6.3 - 8.2 g/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Alkaline Phosphatase	86	38 - 126 Units/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
AST	25	14 - 36 Units/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/29/2024 6:52 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 6:07 AM PST

Narrative

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MERCY LAB CV - 01/29/2024 6:52 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

HIV ANTIGEN/ANTIBODY COMBO - Final result (01/28/2024 8:28 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HIV Antigen / Antibody Combo	Non Reactive	Non Reactive		01/28/2024 10:45 PM PST	SCRIPPS LAB	

Comment: HIV-1 p24 antigen and HIV-1/HIV-2 antibodies are not detected.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 8:28 PM PST	01/28/2024 8:31 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

CYCLIC CITRUL PEPTIDE AB, IGG (CYCLIC CITRUL PEPTIDE ANTIBODY, IGG) - Final result (01/28/2024 12:19 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cyclic Citrul Peptide Antibodies, IgG	1.0	<5.0 U/mL		01/28/2024 6:25 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 12:19 PM PST	01/28/2024 12:58 PM PST

Narrative

SCRIPPS LAB - 01/28/2024 6:25 PM PST
Alinity i Anti-CCP results should not be used interchangeably with other manufacturers' methods for anti-CCP determinations.

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

RHEUMATOID FACTOR, QUANTITATIVE - Final result (01/28/2024 12:19 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Rheumatoid Factor	25	<30 U/mL		01/28/2024 5:53 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 12:19 PM PST	01/28/2024 12:58 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

ANA (ANTI-NUCLEAR ANTIBODY) - Final result (01/28/2024 12:19 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ANA	Negative	Negative		01/29/2024 11:07 AM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 12:19 PM PST	01/28/2024 12:58 PM PST

Narrative

SCRIPPS LAB - 01/29/2024 11:07 AM PST
Performed using Bio-Rad Autoimmune qualitative enzyme immunoassay (EIA).

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

GLUCOSE, POC - Final result (01/28/2024 7:25 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	130	70 - 125 mg/dL		01/28/2024 7:27 AM PST	MERCY LAB CV	
Operator ID	126,288			01/28/2024 7:27 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/28/2024 7:25 AM PST	01/28/2024 7:27 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

IRON +TIBC (IRON AND TIBC) - Final result (01/28/2024 5:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Iron	37	50 - 170 mcg/dL		01/28/2024 5:58 PM PST	SCRIPPS LAB	
Transferrin	123	180 - 382 mg/dL		01/28/2024 5:58 PM PST	SCRIPPS LAB	
Iron Binding Capacity Calculated	154	250 - 450 mcg/dL		01/28/2024 5:58 PM PST	SCRIPPS LAB	
Iron Saturation	24	20 - 50 %		01/28/2024 5:58 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 5:40 AM PST	01/28/2024 6:10 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

FERRITIN - Final result (01/28/2024 5:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Ferritin	113.52	4.63 - 204.00 ng/mL		01/28/2024 6:14 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 5:40 AM PST	01/28/2024 6:10 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

COMPREHENSIVE METABOLIC PANEL - Final result (01/28/2024 5:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/28/2024 7:00 AM PST	MERCY LAB CV	
Glucose	97	70 - 125 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Sodium	130	137 - 145 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Chloride	94	98 - 107 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Creatinine	0.67	0.50 - 1.00 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	101	>60 mL/min/1.73m*2		01/28/2024 7:00 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Osmolality Calculated	270	280 - 305 mOsm/kg H20		01/28/2024 7:00 AM PST	MERCY LAB CV	
Calcium	8.4	8.4 - 10.2 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Protein, total	7.4	6.3 - 8.2 g/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Albumin	2.9	3.5 - 5.0 g/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Alkaline Phosphatase	81	38 - 126 Units/L		01/28/2024 7:00 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
AST	32	14 - 36 Units/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 5:40 AM PST	01/28/2024 6:10 AM PST

Narrative

MERCY LAB CV - 01/28/2024 7:00 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

HEMOGRAM - Final result (01/27/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.8	3.4 - 11.0 K/mcL		01/27/2024 7:03 AM PST	MERCY LAB CV	
RBC	2.42	3.98 - 5.25 M/mcL		01/27/2024 7:03 AM PST	MERCY LAB CV	
HGB	7.6	11.9 - 15.3 g/dL		01/27/2024 7:03 AM PST	MERCY LAB CV	
HCT	24.3	37.3 - 46.7 %		01/27/2024 7:03 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/27/2024 7:03 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/27/2024 7:03 AM PST	MERCY LAB CV	
MCHC	31.3	31.0 - 36.0 g/dL		01/27/2024 7:03 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/27/2024 7:03 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	452	150 - 425 K/mcL		01/27/2024 7:03 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/27/2024 5:53 AM PST	01/27/2024 6:50 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/27/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/27/2024 7:36 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Sodium	129	137 - 145 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Chloride	97	98 - 107 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Creatinine	0.63	0.50 - 1.00 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	102	>60 mL/min/1.73m*2		01/27/2024 7:36 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Osmolality Calculated	269	280 - 305 mOsm/kg H20		01/27/2024 7:36 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Protein, total	6.7	6.3 - 8.2 g/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Albumin	2.7	3.5 - 5.0 g/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Alkaline Phosphatase	71	38 - 126 Units/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
AST	26	14 - 36 Units/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/27/2024 5:53 AM PST	01/27/2024 6:48 AM PST

Narrative

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MERCY LAB CV - 01/27/2024 7:36 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/26/2024 12:06 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	131	70 - 125 mg/dL		01/26/2024 12:09 PM PST	MERCY LAB CV	
Operator ID	255,308			01/26/2024 12:09 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/26/2024 12:06 PM PST	01/26/2024 12:09 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/26/2024 10:32 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/31/2024 12:09 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/26/2024 10:32 AM PST	01/26/2024 11:13 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/26/2024 10:23 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/31/2024 11:08 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of part of right upper limb / Unknown	Venipuncture / Unknown	01/26/2024 10:23 AM PST	01/26/2024 10:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MRI cervical spine with and without contrast (MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST) - Final result (01/26/2024 10:08 AM PST)

Anatomical Region	Laterality	Modality
C-spine		Magnetic Resonance

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/26/2024 10:14 AM PST	

Impressions

01/26/2024 10:23 AM PST

Impressions

Impression:

Contrast imaging is very limited due to extensive motion.

Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema.

Findings are concerning for discitis/osteomyelitis at C4-C5.

release

Narrative

01/26/2024 10:23 AM PST

EXAMINATION: MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST

DATE/TIME: 1/26/2024 10:08 AM

HISTORY: concern for osteomyelitis from previous MRI without contrast

COMPARISON: Noncontrast MRI of the cervical spine performed 1 day prior.

TECHNIQUE: Multiplanar multisequence MRI examination was performed without and with contrast.

FINDINGS:

The exam is very limited due to extensive motion artifact.

Pre and postcontrast axial and postcontrast sagittal images were obtained.

There is a large amount of prevertebral edema with generalized enhancement.
This extends into the chest, anterior mediastinum, series 6 images 10 through 13. No discrete fluid collection.

There is also enhancement of the C4, C5 and C6 vertebral bodies with enhancement of the C4-C5 disc. The other discs are difficult to visualize due to severe disc height loss at C3-4, C5-6 and C6-7.

No definite abnormal enhancement within the spinal canal, epidural compartment..

Procedure Note

Anila Jonnala, MD - 01/26/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST

DATE/TIME: 1/26/2024 10:08 AM

HISTORY: concern for osteomyelitis from previous MRI without contrast

COMPARISON: Noncontrast MRI of the cervical spine performed 1 day prior.

TECHNIQUE: Multiplanar multisequence MRI examination was performed without and with contrast.

FINDINGS:

The exam is very limited due to extensive motion artifact.

Pre and postcontrast axial and postcontrast sagittal images were obtained.

There is a large amount of prevertebral edema with generalized enhancement.
This extends into the chest, anterior mediastinum, series 6 images 10 through 13. No discrete fluid collection.

There is also enhancement of the C4, C5 and C6 vertebral bodies with enhancement of the C4-C5 disc. The other discs are difficult to visualize due to severe disc height loss at C3-4, C5-6 and C6-7.

No definite abnormal enhancement within the spinal canal, epidural compartment..

IMPRESSION:
Impression:

Contrast imaging is very limited due to extensive motion.

Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema.

Findings are concerning for discitis/osteomyelitis at C4-C5.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG MRI PROCEDURES	Final Result

HEMOGRAM - Final result (01/26/2024 5:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.7	3.4 - 11.0 K/mcL		01/26/2024 7:56 AM PST	MERCY LAB CV	
RBC	2.67	3.98 - 5.25 M/mcL		01/26/2024 7:56 AM PST	MERCY LAB CV	
HGB	8.2	11.9 - 15.3 g/dL		01/26/2024 7:56 AM PST	MERCY LAB CV	
HCT	26.7	37.3 - 46.7 %		01/26/2024 7:56 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/26/2024 7:56 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/26/2024 7:56 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCHC	30.7	31.0 - 36.0 g/dL		01/26/2024 7:56 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/26/2024 7:56 AM PST	MERCY LAB CV	
Platelet Count	478	150 - 425 K/mcL		01/26/2024 7:56 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/26/2024 5:36 AM PST	01/26/2024 7:19 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/26/2024 5:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/26/2024 7:40 AM PST	MERCY LAB CV	
Glucose	100	70 - 125 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Chloride	98	98 - 107 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Creatinine	0.67	0.50 - 1.00 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	101	>60 mL/min/1.73m*2		01/26/2024 7:40 AM PST	MERCY LAB CV	
Anion Gap	1	6 - 14 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Osmolality Calculated	273	280 - 305 mOsm/kg H20		01/26/2024 7:40 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Calcium	8.4	8.4 - 10.2 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Protein, total	6.9	6.3 - 8.2 g/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Alkaline Phosphatase	89	38 - 126 Units/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
AST	26	14 - 36 Units/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
ALT	14	<=34 Units/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/26/2024 5:36 AM PST	01/26/2024 7:20 AM PST

Narrative

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MERCY LAB CV - 01/26/2024 7:40 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (01/25/2024 10:52 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	131	<=30 mm/hr		01/26/2024 12:38 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 10:52 PM PST	01/25/2024 10:54 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (01/25/2024 10:52 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	32.7	<10.0 mg/L		01/25/2024 11:11 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 10:52 PM PST	01/25/2024 10:54 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MRI cervical spine without contrast (MRI CERVICAL SPINE WITHOUT CONTRAST) - Final result (01/25/2024 6:33 PM PST)

Anatomical Region	Laterality	Modality
C-spine		Magnetic Resonance

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/25/2024 6:38 PM PST	

Impressions

01/25/2024 9:48 PM PST
IMPRESSION:
1. Abnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation.
2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.

Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024.

release 1419

A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

Narrative

01/25/2024 9:48 PM PST

Narrative

EXAMINATION: MRI CERVICAL SPINE WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:33 PM

HISTORY: Evaluate severe ongoing debilitating neck pain. Cough, fever and neck pain.

COMPARISON: None.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI of the cervical spine was performed without contrast.

FINDINGS:

Abnormal edema at the vertebral bodies of C4, C5, C6 with indistinct anterior cortex at C4, C6.
Abnormal prevertebral collection, fluid from C2-C7. Fluid in the disc space of C4-C5.
No epidural collection.
Mild posterior disc bulge C4-T4 identified. There is moderate canal narrowing from disc bulge and short pedicles at C4-C7. Mild central canal narrowing C7-T1, T1-T2.
Moderate facet arthrosis identified. Axial images are motion degraded.

Severe right neural foramen narrowing C4-C5, moderate neural foramen narrowing bilaterally C5-C6, C6-C7.

Procedure Note

David Vu, MD - 01/25/2024

Formatting of this note might be different from the original.

EXAMINATION: MRI CERVICAL SPINE WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:33 PM

HISTORY: Evaluate severe ongoing debilitating neck pain. Cough, fever and neck pain.

COMPARISON: None.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI of the cervical spine was performed without contrast.

FINDINGS:

Abnormal edema at the vertebral bodies of C4, C5, C6 with indistinct anterior cortex at C4, C6.
Abnormal prevertebral collection, fluid from C2-C7. Fluid in the disc space of C4-C5.
No epidural collection.
Mild posterior disc bulge C4-T4 identified. There is moderate canal narrowing from disc bulge and short pedicles at C4-C7. Mild central canal narrowing C7-T1, T1-T2.
Moderate facet arthrosis identified. Axial images are motion degraded.

Severe right neural foramen narrowing C4-C5, moderate neural foramen narrowing bilaterally C5-C6, C6-C7.

IMPRESSION:
IMPRESSION:
1. Abnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation.
2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.

Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024.

release 1419

A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG MRI PROCEDURES	Final Result

MRI brain without contrast (MRI BRAIN WITHOUT CONTRAST) - Final result (01/25/2024 6:07 PM PST)

Anatomical Region	Laterality	Modality
Head		Magnetic Resonance

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/25/2024 6:10 PM PST	

Impressions

01/25/2024 9:48 PM PST

IMPRESSION:
1. No acute intracranial findings.
2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change.
3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process.

release

Narrative

01/25/2024 9:48 PM PST

EXAMINATION: MRI BRAIN WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:07 PM

HISTORY: Severe occipital and neck headaches. Shortness of breath, pleural effusion.

COMPARISON: CT chest same day.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI examination was performed without contrast.

FINDINGS:
There is no acute infarct.

There is no intracranial hemorrhage. Punctate susceptibility at the left caudate body is probably from mineralization. Minimal FLAIR high signal in the white matter is likely from microvascular change. This is age-appropriate. The flow voids are intact.

Small amount of fluid in the right mastoid air cells. Mild mucosal thickening of the ethmoid air cells. The soft tissues are unremarkable.

Low signal within the diploic space, upper cervical spine from marrow replacing process or anemia.
Moderate degenerative change in the upper cervical spine at C3-C4, C4-C5. Moderate facet arthrosis in the upper cervical spine.
Moderate central canal narrowing C4-C5.

Procedure Note

David Vu, MD - 01/25/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: MRI BRAIN WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:07 PM

HISTORY: Severe occipital and neck headaches. Shortness of breath, pleural effusion.

COMPARISON: CT chest same day.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI examination was performed without contrast.

FINDINGS:
There is no acute infarct.

There is no intracranial hemorrhage. Punctate susceptibility at the left caudate body is probably from mineralization. Minimal FLAIR high signal in the white matter is likely from microvascular change. This is age-appropriate. The flow voids are intact.

Small amount of fluid in the right mastoid air cells. Mild mucosal thickening of the ethmoid air cells. The soft tissues are unremarkable.

Low signal within the diploic space, upper cervical spine from marrow replacing process or anemia.
Moderate degenerative change in the upper cervical spine at C3-C4, C4-C5. Moderate facet arthrosis in the upper cervical spine. Moderate central canal narrowing C4-C5.

IMPRESSION:
IMPRESSION:
1. No acute intracranial findings.
2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change.
3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG MRI PROCEDURES	Final Result

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/25/2024 1:14 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/25/2024 1:31 PM PST	

Impressions

01/25/2024 1:31 PM PST
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated

release

Narrative

01/25/2024 1:31 PM PST

Narrative

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EXAMINATION:  X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME:  1/25/2024 1:14 PM

HISTORY:  status post Right thoracentesis

COMPARISON: 1/22/2024

FINDINGS:

LUNGS: Basilar right compressive change with volume loss
VASCULATURE:  Normal. Unremarkable pulmonary vasculature.
CARDIAC:  Normal. No cardiac silhouette abnormality.
MEDIASTINUM:  Normal. No visible mass or adenopathy.
PLEURA: Ongoing moderate to large right effusion. No pneumothorax
BONES:  Normal. No fracture or visible bony lesion.
OTHER:  Negative.
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Procedure Note

Trevor Nelson, MD - 01/25/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/25/2024 1:14 PM

HISTORY: status post Right thoracentesis

COMPARISON: 1/22/2024

FINDINGS:

LUNGS: Basilar right compressive change with volume loss
VASCULATURE: Normal. Unremarkable pulmonary vasculature.
CARDIAC: Normal. No cardiac silhouette abnormality.
MEDIASTINUM: Normal. No visible mass or adenopathy.
PLEURA: Ongoing moderate to large right effusion. No pneumothorax
BONES: Normal. No fracture or visible bony lesion.
OTHER: Negative.

IMPRESSION:
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

PH, PLEURAL FLUID - Final result (01/25/2024 10:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Pleural Fluid	7.27	Not Established		01/25/2024 10:51 AM PST	MERCY LAB CV	
Operator ID	202,545			01/25/2024 10:51 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:35 AM PST	01/25/2024 10:43 AM PST

Narrative

MERCY LAB CV - 01/25/2024 10:51 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PATHOLOGIST FLUID COMMENT - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Pathologist Review, CSF/ Body Fluids	Morphologic analysis of the smear shows severe acute inflammation with PMN vacuoles without atypia. Carla Stayboldt, M.D.			01/25/2024 4:18 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, WOUND (WOUND/FLUID/TISSUE CULTURE W/STAIN) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Wound Culture	No Growth Final		MIC	01/29/2024 7:46 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	Many WBCs			01/29/2024 7:46 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	No organisms seen			01/29/2024 7:46 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

BODY FLUID CELL COUNT WITH DIFF (BODY FLUID CELL COUNT WITH DIFFERENTIAL) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Volume (ML)	265.0	mL		01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Color	Orange			01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Appearance	Slightly Hazy			01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Nucleated Cell Count	6,055	/mcL		01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid RBC Count	17,000	/mcL		01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Neutrophils %	81	%		01/25/2024 2:42 PM PST	MERCY LAB CV	
Comment: Appended report. These results have been appended to a previously preliminary verified report.						
Fluid Lymphocytes %	10	%		01/25/2024 2:42 PM PST	MERCY LAB CV	
Comment: Appended report. These results have been appended to a previously preliminary verified report.						
Fluid Mononuclears %	9	%		01/25/2024 2:42 PM PST	MERCY LAB CV	
Comment: Appended report. These results have been appended to a previously preliminary verified report.						

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 2:42 PM PST
Reference Interval has not been established for this specimen type.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTEIN BODY FLUID (PROTEIN, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/25/2024 12:25 PM PST	MERCY LAB CV	
Protein (FL)	4.3	Not Established g/dL		01/25/2024 12:25 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

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MERCY LAB CV - 01/25/2024 12:25 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories.
The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for clinical purposes.
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Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTATE DEHYDROGENASE BODY FLUID (LACTATE DEHYDROGENASE, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/25/2024 1:52 PM PST	MERCY LAB CV	
Lactate Dehrdrogenase (FL)	1,862	Not Established Units/L		01/25/2024 1:52 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

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MERCY LAB CV - 01/25/2024 1:52 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories.
The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for clinical purposes.
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Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE BODY FLUID (GLUCOSE, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/25/2024 12:25 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose (FL)	65	Not Established mg/dL		01/25/2024 12:25 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 12:25 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

ALBUMIN BODY FLUID (ALBUMIN, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Albumin (FL)	1.7	Not Established g/dL		01/25/2024 12:25 PM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/25/2024 12:25 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 12:25 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

HEMOGRAM - Final result (01/25/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.6	3.4 - 11.0 K/mcL		01/25/2024 6:26 AM PST	MERCY LAB CV	
RBC	2.54	3.98 - 5.25 M/mcL		01/25/2024 6:26 AM PST	MERCY LAB CV	
HGB	7.8	11.9 - 15.3 g/dL		01/25/2024 6:26 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HCT	25.4	37.3 - 46.7 %		01/25/2024 6:26 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/25/2024 6:26 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/25/2024 6:26 AM PST	MERCY LAB CV	
MCHC	30.7	31.0 - 36.0 g/dL		01/25/2024 6:26 AM PST	MERCY LAB CV	
RDW	13.1	11.7 - 14.9 %		01/25/2024 6:26 AM PST	MERCY LAB CV	
Platelet Count	413	150 - 425 K/mcL		01/25/2024 6:26 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 5:31 AM PST	01/25/2024 6:01 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTATE DEHYDROGENASE (LACTATE DEHYDROGENASE, TOTAL) - Final result (01/25/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Lactate Dehydrogenase	141	120 - 246 Units/L		01/25/2024 6:27 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 5:31 AM PST	01/25/2024 5:59 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/25/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/25/2024 6:28 AM PST	MERCY LAB CV	
Glucose	120	70 - 125 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Chloride	97	98 - 107 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/25/2024 6:28 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Osmolality Calculated	276	280 - 305 mOsm/kg H20		01/25/2024 6:28 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Protein, total	6.7	6.3 - 8.2 g/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Alkaline Phosphatase	74	38 - 126 Units/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
AST	27	14 - 36 Units/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 5:31 AM PST	01/25/2024 5:59 AM PST

Narrative

MERCY LAB CV - 01/25/2024 6:28 AM PST

Narrative

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for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/24/2024 5:48 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.8	3.4 - 11.0 K/mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
RBC	2.58	3.98 - 5.25 M/mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
HGB	8.0	11.9 - 15.3 g/dL		01/24/2024 6:30 AM PST	MERCY LAB CV	
HCT	26.3	37.3 - 46.7 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/24/2024 6:30 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/24/2024 6:30 AM PST	MERCY LAB CV	
MCHC	30.4	31.0 - 36.0 g/dL		01/24/2024 6:30 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Platelet Count	364	150 - 425 K/mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
MPV	8.6	9.0 - 12.8 fL		01/24/2024 6:30 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/24/2024 6:30 AM PST	MERCY LAB CV	
Neutrophils %	65.7	38.0 - 74.0 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
IG%	0.4	0.0 - 1.2 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Lymphocytes %	19.3	16.0 - 48.0 %		01/24/2024 6:30 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	11.1	4.9 - 12.5 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Eosinophils %	2.7	0.0 - 9.5 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.13	1.50 - 7.40 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Lymphocytes	0.92	0.90 - 3.10 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Monocytes	0.53	0.26 - 0.87 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.13	0.00 - 0.51 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Preliminary Abs Neut	3.13	1.50 - 7.40 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/24/2024 5:48 AM PST	01/24/2024 6:21 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/24/2024 5:48 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/24/2024 6:58 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Potassium	4.9	3.5 - 5.1 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	98	98 - 107 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/24/2024 6:58 AM PST	MERCY LAB CV	
Anion Gap	-3	6 - 14 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Osmolality Calculated	275	280 - 305 mOsm/kg H20		01/24/2024 6:58 AM PST	MERCY LAB CV	
Calcium	8.0	8.4 - 10.2 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Protein, total	6.6	6.3 - 8.2 g/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Alkaline Phosphatase	74	38 - 126 Units/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
AST	25	14 - 36 Units/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
ALT	17	<=34 Units/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/24/2024 5:48 AM PST	01/24/2024 6:23 AM PST

Narrative

MERCY LAB CV - 01/24/2024 6:58 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/23/2024 5:44 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.2	3.4 - 11.0 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
RBC	2.52	3.98 - 5.25 M/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
HGB	7.8	11.9 - 15.3 g/dL		01/23/2024 6:58 AM PST	MERCY LAB CV	
HCT	25.6	37.3 - 46.7 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/23/2024 6:58 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/23/2024 6:58 AM PST	MERCY LAB CV	
MCHC	30.5	31.0 - 36.0 g/dL		01/23/2024 6:58 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Platelet Count	353	150 - 425 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
MPV	8.8	9.0 - 12.8 fL		01/23/2024 6:58 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/23/2024 6:58 AM PST	MERCY LAB CV	
Neutrophils %	57.0	38.0 - 74.0 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
IG%	0.7	0.0 - 1.2 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Lymphocytes %	25.2	16.0 - 48.0 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Monocytes %	14.2	4.9 - 12.5 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Eosinophils %	2.2	0.0 - 9.5 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.37	1.50 - 7.40 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	1.05	0.90 - 3.10 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Monocytes	0.59	0.26 - 0.87 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.09	0.00 - 0.51 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.37	1.50 - 7.40 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/23/2024 5:44 AM PST	01/23/2024 6:20 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/23/2024 5:44 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/23/2024 6:46 AM PST	MERCY LAB CV	
Glucose	92	70 - 125 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Sodium	128	137 - 145 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Potassium	4.1	3.5 - 5.1 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		01/23/2024 6:46 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Anion Gap	-5	6 - 14 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Osmolality Calculated	267	280 - 305 mOsm/kg H20		01/23/2024 6:46 AM PST	MERCY LAB CV	
Calcium	7.8	8.4 - 10.2 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Protein, total	6.3	6.3 - 8.2 g/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Albumin	2.5	3.5 - 5.0 g/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Alkaline Phosphatase	68	38 - 126 Units/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
AST	25	14 - 36 Units/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
ALT	14	<=34 Units/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/23/2024 5:44 AM PST	01/23/2024 6:20 AM PST

Narrative

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MERCY LAB CV - 01/23/2024 6:46 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/22/2024 12:09 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/22/2024 12:25 PM PST	

Impressions

01/22/2024 12:26 PM PST

IMPRESSION:
1. Right chest tube removal with no pneumothorax. There is stable pleural-parenchymal opacity at the right lung base.

2. Mild left basilar atelectatic versus low-grade inflammatory change.

release
CC Dr: p: f:

Narrative

01/22/2024 12:26 PM PST

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/22/2024 12:09 PM

HISTORY: dyspnea

COMPARISON: 1/21/2024

FINDINGS: Interval removal of the right chest tube. There is continued moderate pleural thickening at the right lung base with associated mild to moderate patchy parenchymal opacity and streak-like change. Minimal streak-like changes are seen at the left lung base with minimal blunting of left lateral costophrenic angle.

Stable mild cardiomegaly. No pneumothorax or pulmonary venous hypertension.

Procedure Note

Ross Christensen, MD - 01/22/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/22/2024 12:09 PM

HISTORY: dyspnea

COMPARISON: 1/21/2024

FINDINGS: Interval removal of the right chest tube. There is continued moderate pleural thickening at the right lung base with associated mild to moderate patchy parenchymal opacity and streak-like change. Minimal streak-like changes are seen at the left lung base with minimal blunting of left lateral costophrenic angle.

Stable mild cardiomegaly. No pneumothorax or pulmonary venous hypertension.

IMPRESSION:
IMPRESSION:
1. Right chest tube removal with no pneumothorax. There is stable pleural-parenchymal opacity at the right lung base.

2. Mild left basilar atelectatic versus low-grade inflammatory change.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Diep Dao DO	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/22/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.8	3.4 - 11.0 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
RBC	2.69	3.98 - 5.25 M/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
HGB	8.4	11.9 - 15.3 g/dL		01/22/2024 6:44 AM PST	MERCY LAB CV	
HCT	26.8	37.3 - 46.7 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/22/2024 6:44 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/22/2024 6:44 AM PST	MERCY LAB CV	
MCHC	31.3	31.0 - 36.0 g/dL		01/22/2024 6:44 AM PST	MERCY LAB CV	
RDW	13.3	11.7 - 14.9 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Platelet Count	368	150 - 425 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
MPV	8.9	9.0 - 12.8 fL		01/22/2024 6:44 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/22/2024 6:44 AM PST	MERCY LAB CV	
Neutrophils %	67.6	38.0 - 74.0 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
IG%	0.4	0.0 - 1.2 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Lymphocytes %	17.6	16.0 - 48.0 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Monocytes %	12.2	4.9 - 12.5 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Eosinophils %	1.2	0.0 - 9.5 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.27	1.50 - 7.40 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Lymphocytes	0.85	0.90 - 3.10 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Monocytes	0.59	0.26 - 0.87 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.06	0.00 - 0.51 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Preliminary Abs Neut	3.27	1.50 - 7.40 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/22/2024 5:31 AM PST	01/22/2024 5:56 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/22/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/22/2024 7:13 AM PST	MERCY LAB CV	
Glucose	105	70 - 125 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
CO2, Total	35	22 - 30 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Urea Nitrogen	15	7 - 17 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Creatinine	0.86	0.50 - 1.00 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	78	>60 mL/min/1.73m*2		01/22/2024 7:13 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality Calculated	273	280 - 305 mOsm/kg H20		01/22/2024 7:13 AM PST	MERCY LAB CV	
Calcium	7.9	8.4 - 10.2 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Protein, total	5.9	6.3 - 8.2 g/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Albumin	2.4	3.5 - 5.0 g/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Alkaline Phosphatase	61	38 - 126 Units/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
AST	23	14 - 36 Units/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
ALT	11	<=34 Units/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/22/2024 5:31 AM PST	01/22/2024 5:56 AM PST

Narrative

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MERCY LAB CV - 01/22/2024 7:13 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SODIUM - Final result (01/21/2024 3:51 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	126	137 - 145 mmol/L		01/21/2024 4:07 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 3:51 PM PST	01/21/2024 3:55 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

OSMOLALITY - Final result (01/21/2024 3:51 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality	281	275 - 295 mOsm/kg H2O		01/21/2024 7:57 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 3:51 PM PST	01/21/2024 3:55 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Jennifer Pham MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/21/2024 5:43 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/21/2024 9:32 AM PST	

Impressions

01/21/2024 9:34 AM PST
IMPRESSION:

Right pleural catheter in place.

Small to moderate pleural effusion and patchy right basilar opacity, probably atelectasis.

No pneumothorax.

release

Narrative

01/21/2024 9:34 AM PST

Narrative

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EXAMINATION:  X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME:  1/21/2024 5:43 AM

HISTORY:  Respiratory failure, not elsewhere classified

COMPARISON: 1/20/2024.

FINDINGS:

There is a right pleural catheter in place. A small to moderate right pleural effusion is not
significantly changed. There is no pneumothorax. Patchy right basilar opacity is noted.

The left lung field is clear. There is a trace left pleural effusion.

The cardiac silhouette is magnified.
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Procedure Note

Tomer Roth, MD - 01/21/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/21/2024 5:43 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/20/2024.

FINDINGS:

There is a right pleural catheter in place. A small to moderate right pleural effusion is not significantly changed. There is no pneumothorax. Patchy right basilar opacity is noted.

The left lung field is clear. There is a trace left pleural effusion.

The cardiac silhouette is magnified.

IMPRESSION:
IMPRESSION:

Right pleural catheter in place.

Small to moderate pleural effusion and patchy right basilar opacity, probably atelectasis.

No pneumothorax.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.7	3.4 - 11.0 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
RBC	2.74	3.98 - 5.25 M/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
HGB	8.6	11.9 - 15.3 g/dL		01/21/2024 6:00 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HCT	27.4	37.3 - 46.7 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/21/2024 6:00 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/21/2024 6:00 AM PST	MERCY LAB CV	
MCHC	31.4	31.0 - 36.0 g/dL		01/21/2024 6:00 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Platelet Count	404	150 - 425 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
MPV	8.9	9.0 - 12.8 fL		01/21/2024 6:00 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/21/2024 6:00 AM PST	MERCY LAB CV	
Neutrophils %	67.4	38.0 - 74.0 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
IG%	0.7	0.0 - 1.2 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Lymphocytes %	19.9	16.0 - 48.0 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Monocytes %	10.0	4.9 - 12.5 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Eosinophils %	1.1	0.0 - 9.5 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Basophils %	0.9	0.0 - 1.6 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.83	1.50 - 7.40 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.13	0.90 - 3.10 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Monocytes	0.57	0.26 - 0.87 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.06	0.00 - 0.51 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Preliminary Abs Neut	3.83	1.50 - 7.40 K/ mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CYCLIC CITRUL PEPTIDE AB, IGG (CYCLIC CITRUL PEPTIDE ANTIBODY, IGG) - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cyclic Citrul Peptide Antibodies, IgG	0.7	<5.0 U/ mL		01/21/2024 6:17 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:45 AM PST

Narrative

SCRIPPS LAB - 01/21/2024 6:17 PM PST
Alinity i Anti-CCP results should not be used interchangeably with other manufacturers' methods for anti-CCP determinations.

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

RHEUMATOID FACTOR, QUANTITATIVE - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Rheumatoid Factor	19	<30 U/ mL		01/21/2024 6:28 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:47 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

ANA (ANTI-NUCLEAR ANTIBODY) - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ANA	Negative	Negative		01/22/2024 11:18 AM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:47 AM PST

Narrative

SCRIPPS LAB - 01/22/2024 11:18 AM PST
Performed using Bio-Rad Autoimmune qualitative enzyme immunoassay (EIA).

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

COMPREHENSIVE METABOLIC PANEL - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/21/2024 6:08 AM PST	MERCY LAB CV	
Glucose	129	70 - 125 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Sodium	127	137 - 145 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Urea Nitrogen	10	7 - 17 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/21/2024 6:08 AM PST	MERCY LAB CV	
Anion Gap	-5	6 - 14 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Osmolality Calculated	265	280 - 305 mOsm/kg H20		01/21/2024 6:08 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Calcium	8.1	8.4 - 10.2 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Protein, total	6.0	6.3 - 8.2 g/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Albumin	2.4	3.5 - 5.0 g/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Alkaline Phosphatase	57	38 - 126 Units/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
AST	21	14 - 36 Units/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:45 AM PST

Narrative

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MERCY LAB CV - 01/21/2024 6:08 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

OSMOLALITY,URINE (OSMOLALITY, URINE) - Final result (01/20/2024 3:47 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality (UR)	152	50 - 1,200 mOsm/kg H2O		01/20/2024 4:43 PM PST	MERCY LAB CV	

Comment: Note: New Reference Range

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/20/2024 3:47 PM PST	01/20/2024 4:24 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SODIUM, RANDOM URINE - Final result (01/20/2024 3:46 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium, (Ur)	17	Not Established mmol/L		01/20/2024 4:38 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Voided urine specimen / Unknown		01/20/2024 3:46 PM PST	01/20/2024 4:24 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CREATININE, RANDOM URINE - Final result (01/20/2024 3:46 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Creatinine, (UR)	15.62	Not Established mg/dL		01/20/2024 4:38 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Voided urine specimen / Unknown		01/20/2024 3:46 PM PST	01/20/2024 4:24 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/20/2024 6:48 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/20/2024 8:06 AM PST	

Impressions

01/20/2024 8:07 AM PST

Impressions

IMPRESSION: Right-sided pigtail chest tube remains in place. There is minimal basilar volume loss. Cardiomegaly again noted. Mild redistribution.

release
CC Dr: p: f:

Narrative

01/20/2024 8:07 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 6:48 AM

HISTORY: Respiratory failure.

COMPARISON: 0112 hours.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

Procedure Note

Richard Abello, MD - 01/20/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 6:48 AM

HISTORY: Respiratory failure.

COMPARISON: 0112 hours.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

IMPRESSION:
IMPRESSION: Right-sided pigtail chest tube remains in place. There is minimal basilar volume loss. Cardiomegaly again noted. Mild redistribution.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/20/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	7.6	3.4 - 11.0 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
RBC	3.12	3.98 - 5.25 M/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
HGB	9.8	11.9 - 15.3 g/dL		01/20/2024 6:07 AM PST	MERCY LAB CV	
HCT	31.6	37.3 - 46.7 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		01/20/2024 6:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCH	31	26 - 33 pg		01/20/2024 6:07 AM PST	MERCY LAB CV	
MCHC	31.0	31.0 - 36.0 g/dL		01/20/2024 6:07 AM PST	MERCY LAB CV	
RDW	13.6	11.7 - 14.9 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Platelet Count	466	150 - 425 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
MPV	9.0	9.0 - 12.8 fL		01/20/2024 6:07 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/20/2024 6:07 AM PST	MERCY LAB CV	
Neutrophils %	73.8	38.0 - 74.0 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Lymphocytes %	15.5	16.0 - 48.0 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Monocytes %	8.7	4.9 - 12.5 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Eosinophils %	0.5	0.0 - 9.5 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	5.61	1.50 - 7.40 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
IG#	0.06	0.00 - 0.10 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.18	0.90 - 3.10 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.66	0.26 - 0.87 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.04	0.00 - 0.51 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	5.61	1.50 - 7.40 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/20/2024 5:35 AM PST	01/20/2024 6:01 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/20/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/20/2024 6:42 AM PST	MERCY LAB CV	
Glucose	96	70 - 125 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Sodium	129	137 - 145 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Potassium	4.9	3.5 - 5.1 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
CO2, Total	35	22 - 30 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Urea Nitrogen	12	7 - 17 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Creatinine	0.66	0.50 - 1.00 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	101	>60 mL/min/1.73m*2		01/20/2024 6:42 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Osmolality Calculated	268	280 - 305 mOsm/kg H20		01/20/2024 6:42 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Protein, total	6.2	6.3 - 8.2 g/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Albumin	2.5	3.5 - 5.0 g/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Alkaline Phosphatase	64	38 - 126 Units/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
AST	18	14 - 36 Units/L		01/20/2024 6:42 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ALT	12	<=34 Units/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/20/2024 5:35 AM PST	01/20/2024 6:01 AM PST

Narrative

MERCY LAB CV - 01/20/2024 6:42 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/20/2024 1:22 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/20/2024 8:50 AM PST	

Impressions

01/20/2024 8:51 AM PST
IMPRESSION: Stable position of right chest tube, some loculated pleural fluid and no pneumothorax. Mild bibasilar volume loss.

These findings are concordant with the preliminary interpretation provided by StatRad radiologist,

Statrad Code: A

release
CC Dr: p: f:

Narrative

01/20/2024 8:51 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 1:22 AM

HISTORY: Pain/cough/SOB/infiltrate.

COMPARISON: Prior day.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

Procedure Note

Richard Abello, MD - 01/20/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 1:22 AM

HISTORY: Pain/cough/SOB/infiltrate.

COMPARISON: Prior day.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

IMPRESSION:
IMPRESSION: Stable position of right chest tube, some loculated pleural fluid and no pneumothorax. Mild bibasilar volume loss.

These findings are concordant with the preliminary interpretation provided by StatRad radiologist,

Statrad Code: A

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Jose Pena MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

X-ray foot right 3 or more views (X-RAY FOOT RIGHT 3 OR MORE VIEWS) - Final result (01/19/2024 3:51 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities, Foot	Right	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/19/2024 4:19 PM PST	

Impressions

01/19/2024 4:20 PM PST
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

release

Narrative

01/19/2024 4:20 PM PST
EXAMINATION: X-RAY FOOT RIGHT 3 OR MORE VIEWS

DATE/TIME: 1/19/2024 3:51 PM

HISTORY: pain and swelling.

COMPARISON: None.

FINDINGS:

Chronic erosions involve the distal first metatarsal with sclerotic margins. No acute pathology. No evidence of fracture. No lytic changes.

Hallux valgus.

Procedure Note

Brian Shore, MD - 01/19/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY FOOT RIGHT 3 OR MORE VIEWS

DATE/TIME: 1/19/2024 3:51 PM

HISTORY: pain and swelling.

COMPARISON: None.

FINDINGS:

Chronic erosions involve the distal first metatarsal with sclerotic margins. No acute pathology. No evidence of fracture. No lytic changes.

Hallux valgus.

IMPRESSION:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

release

Authorizing Provider	Result Type	Result Status
Diep Dao DO	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/19/2024 2:25 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/19/2024 2:45 PM PST	

Impressions

01/19/2024 2:46 PM PST
IMPRESSION: No change.

release

Narrative

01/19/2024 2:46 PM PST
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 2:25 PM

HISTORY: Respiratory failure

COMPARISON: 1/19/2024, earlier time.

FINDINGS:

1. Stable position of smallbore right chest tube. No discernible pneumothorax.
2. Persistent small peripheral right pleural effusion.
3. Unchanged haziness in the right lung base.
4. The left lung is clear. Stable cardiomediastinal silhouette.

Procedure Note

Harold Pimenta, MD - 01/19/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 2:25 PM

HISTORY: Respiratory failure

COMPARISON: 1/19/2024, earlier time.

FINDINGS:

1. Stable position of smallbore right chest tube. No discernible pneumothorax.
2. Persistent small peripheral right pleural effusion.
3. Unchanged haziness in the right lung base.
4. The left lung is clear. Stable cardiomediastinal silhouette.

IMPRESSION:
IMPRESSION: No change.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

SURVEILLANCE MRSA CULTURE - Final result (01/19/2024 12:30 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Staph Screen culture	NO Methicillin/ Oxacillin Resistant Staphylococcus aureus		MIC	01/20/2024 2:12 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasal structure / Unknown		01/19/2024 12:30 PM PST	01/19/2024 12:55 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/19/2024 6:38 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/19/2024 7:51 AM PST	

Impressions

01/19/2024 8:17 AM PST

IMPRESSION: No significant change in a right-sided chest tube, a moderate right pleural effusion, right lower lung field infiltrates.

release

Narrative

01/19/2024 8:17 AM PST

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 6:38 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: Chest x-ray 1/18/2024, CT chest 1/16/2024

FINDINGS:
Right-sided chest tube remains with its tip unchanged within the peripheral aspect of the right midlung field.
No significant change in the extent of a moderate right pleural effusion, patchy infiltrate within the right middle lobe and right lower lobe.

The left lung is clear.

Stable cardiomegaly and mediastinal contour.

Procedure Note

Anila Jonnala, MD - 01/19/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 6:38 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: Chest x-ray 1/18/2024, CT chest 1/16/2024

FINDINGS:
Right-sided chest tube remains with its tip unchanged within the peripheral aspect of the right midlung field.
No significant change in the extent of a moderate right pleural effusion, patchy infiltrate within the right middle lobe and right lower lobe.

The left lung is clear.

Stable cardiomegaly and mediastinal contour.

IMPRESSION:
IMPRESSION: No significant change in a right-sided chest tube, a moderate right pleural effusion, right lower lung field infiltrates.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/19/2024 4:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	9.6	3.4 - 11.0 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
RBC	3.21	3.98 - 5.25 M/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
HGB	10.2	11.9 - 15.3 g/dL		01/19/2024 7:09 AM PST	MERCY LAB CV	
HCT	32.8	37.3 - 46.7 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/19/2024 7:09 AM PST	MERCY LAB CV	
MCH	32	26 - 33 pg		01/19/2024 7:09 AM PST	MERCY LAB CV	
MCHC	31.1	31.0 - 36.0 g/dL		01/19/2024 7:09 AM PST	MERCY LAB CV	
RDW	13.7	11.7 - 14.9 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Platelet Count	426	150 - 425 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
MPV				01/19/2024 7:09 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/19/2024 7:09 AM PST	MERCY LAB CV	
Neutrophils %	78.9	38.0 - 74.0 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
IG%	0.9	0.0 - 1.2 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Lymphocytes %	11.4	16.0 - 48.0 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Monocytes %	7.6	4.9 - 12.5 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Eosinophils %	0.7	0.0 - 9.5 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Basophils %	0.5	0.0 - 1.6 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Neutrophils	7.56	1.50 - 7.40 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
IG#	0.09	0.00 - 0.10 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.09	0.90 - 3.10 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Monocytes	0.73	0.26 - 0.87 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.07	0.00 - 0.51 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Preliminary Abs Neut	7.56	1.50 - 7.40 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/19/2024 4:22 AM PST	01/19/2024 5:33 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URIC ACID - Final result (01/19/2024 4:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Uric Acid	2.6	2.5 - 6.2 mg/dL		01/19/2024 12:58 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/19/2024 4:22 AM PST	01/19/2024 5:33 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/19/2024 4:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/19/2024 6:14 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	130	137 - 145 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Potassium	4.7	3.5 - 5.1 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Creatinine	0.60	0.50 - 1.00 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	103	>60 mL/min/1.73m*2		01/19/2024 6:14 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Osmolality Calculated	270	280 - 305 mOsm/kg H20		01/19/2024 6:14 AM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Protein, total	6.2	6.3 - 8.2 g/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Alkaline Phosphatase	62	38 - 126 Units/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
AST	24	14 - 36 Units/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
ALT	13	<=34 Units/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/19/2024 4:22 AM PST	01/19/2024 5:33 AM PST

Narrative

MERCY LAB CV - 01/19/2024 6:14 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/18/2024 9:38 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/18/2024 9:39 AM PST	

Impressions

01/18/2024 9:40 AM PST
IMPRESSION: Stable pleural-parenchymal opacity the right lung base with no pneumothorax.

release
CC Dr: p: f:

Narrative

01/18/2024 9:40 AM PST
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/18/2024 9:38 AM

HISTORY: empyema

COMPARISON: 1/17/2024

FINDINGS: Stable position of the pigtail right chest tube overlying the periphery the right midlung zone. There is ongoing mild right lateral pleural thickening with blunting of the right lateral costophrenic angle, and continued mild hazy and streak-like opacity at the right lung base heart size is at the upper limits of normal with no new mediastinal or hilar abnormality. No pneumothorax.

Procedure Note

Ross Christensen, MD - 01/18/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/18/2024 9:38 AM

HISTORY: empyema

COMPARISON: 1/17/2024

FINDINGS: Stable position of the pigtail right chest tube overlying the periphery the right midlung zone. There is ongoing mild right lateral pleural thickening with blunting of the right lateral costophrenic angle, and continued mild hazy and streak-like opacity at the right lung base heart size is at the upper limits of normal with no new mediastinal or hilar abnormality. No pneumothorax.

IMPRESSION:
IMPRESSION: Stable pleural-parenchymal opacity the right lung base with no pneumothorax.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/18/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	11.6	3.4 - 11.0 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
RBC	3.29	3.98 - 5.25 M/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
HGB	10.1	11.9 - 15.3 g/dL		01/18/2024 7:25 AM PST	MERCY LAB CV	
HCT	34.2	37.3 - 46.7 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
MCV	104	81 - 100 fL		01/18/2024 7:25 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/18/2024 7:25 AM PST	MERCY LAB CV	
MCHC	29.5	31.0 - 36.0 g/dL		01/18/2024 7:25 AM PST	MERCY LAB CV	
RDW	14.0	11.7 - 14.9 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Platelet Count	450	150 - 425 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
MPV	9.5	9.0 - 12.8 fL		01/18/2024 7:25 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
nRBC	0.0	0.0 /100 WBCs		01/18/2024 7:25 AM PST	MERCY LAB CV	
Neutrophils %	80.2	38.0 - 74.0 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
IG%	1.0	0.0 - 1.2 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Lymphocytes %	11.8	16.0 - 48.0 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Monocytes %	6.3	4.9 - 12.5 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Eosinophils %	0.3	0.0 - 9.5 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Basophils %	0.4	0.0 - 1.6 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Neutrophils	9.27	1.50 - 7.40 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
IG#	0.12	0.00 - 0.10 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.36	0.90 - 3.10 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Monocytes	0.73	0.26 - 0.87 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.04	0.00 - 0.51 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Preliminary Abs Neut	9.27	1.50 - 7.40 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/18/2024 5:19 AM PST	01/18/2024 5:38 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/18/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/18/2024 6:06 AM PST	MERCY LAB CV	
Glucose	91	70 - 125 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Creatinine	0.60	0.50 - 1.00 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	103	>60 mL/min/1.73m*2		01/18/2024 6:06 AM PST	MERCY LAB CV	
Anion Gap	1	6 - 14 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Osmolality Calculated	274	280 - 305 mOsm/kg H20		01/18/2024 6:06 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Protein, total	6.5	6.3 - 8.2 g/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Alkaline Phosphatase	70	38 - 126 Units/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
AST	24	14 - 36 Units/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/18/2024 5:19 AM PST	01/18/2024 5:38 AM PST

Narrative

MERCY LAB CV - 01/18/2024 6:06 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (01/17/2024 5:22 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	97	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	97	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	160	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	96	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	338	ms			SCRIPPS RADIOLOGY	
QTC	429	ms			SCRIPPS RADIOLOGY	
P AXIS	63	degrees			SCRIPPS RADIOLOGY	
R AXIS	44	degrees			SCRIPPS RADIOLOGY	
T AXIS	101	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/17/2024 5:22 PM PST	01/25/2024 8:00 AM PST

Impressions

SCRIPPS RADIOLOGY - 01/25/2024 8:00 AM PST
Normal sinus rhythm
Nonspecific T wave abnormality
Abnormal ECG

Narrative

This result has an attachment that is not available.

Procedure Note

Raghava Gollapudi, MD - 01/25/2024
Formatting of this note might be different from the original.
IMPRESSION:
Normal sinus rhythm
Nonspecific T wave abnormality
Abnormal ECG

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/17/2024 6:43 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/17/2024 9:10 AM PST	

Impressions

01/17/2024 9:11 AM PST
IMPRESSION: Stable exam, overall no significant change

release

Narrative

01/17/2024 9:11 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/17/2024 6:43 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/16/2024 chest x-ray and chest CT

FINDINGS:

Right chest tube in place with ongoing small right effusion. No pneumothorax. Stable cardiac contour and pulmonary vessels. Hazy opacity right base.

Procedure Note

Trevor Nelson, MD - 01/17/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/17/2024 6:43 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/16/2024 chest x-ray and chest CT

FINDINGS:

Right chest tube in place with ongoing small right effusion. No pneumothorax. Stable cardiac contour and pulmonary vessels. Hazy opacity right base.

IMPRESSION:
IMPRESSION: Stable exam, overall no significant change

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/17/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	12.8	3.4 - 11.0 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
RBC	3.36	3.98 - 5.25 M/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
HGB	10.4	11.9 - 15.3 g/dL		01/17/2024 6:07 AM PST	MERCY LAB CV	
HCT	34.4	37.3 - 46.7 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/17/2024 6:07 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/17/2024 6:07 AM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		01/17/2024 6:07 AM PST	MERCY LAB CV	
RDW	14.0	11.7 - 14.9 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Platelet Count	509	150 - 425 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
MPV	9.1	9.0 - 12.8 fL		01/17/2024 6:07 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/17/2024 6:07 AM PST	MERCY LAB CV	
Neutrophils %	86.1	38.0 - 74.0 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
IG%	0.9	0.0 - 1.2 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Lymphocytes %	7.6	16.0 - 48.0 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Monocytes %	4.9	4.9 - 12.5 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Eosinophils %	0.2	0.0 - 9.5 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Basophils %	0.3	0.0 - 1.6 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	11.00	1.50 - 7.40 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
IG#	0.12	0.00 - 0.10 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	0.97	0.90 - 3.10 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.62	0.26 - 0.87 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.03	0.00 - 0.51 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	11.00	1.50 - 7.40 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/17/2024 5:19 AM PST	01/17/2024 5:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/17/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/17/2024 6:19 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Urea Nitrogen	19	7 - 17 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/17/2024 6:19 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Anion Gap	1	6 - 14 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Osmolality Calculated	276	280 - 305 mOsm/kg H20		01/17/2024 6:19 AM PST	MERCY LAB CV	
Calcium	8.6	8.4 - 10.2 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Protein, total	6.1	6.3 - 8.2 g/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Albumin	2.5	3.5 - 5.0 g/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Alkaline Phosphatase	77	38 - 126 Units/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
AST	24	14 - 36 Units/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/17/2024 5:19 AM PST	01/17/2024 5:56 AM PST

Narrative

MERCY LAB CV - 01/17/2024 6:19 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS MICROSCOPIC W/ REFLEX TO CULTURE - Final result (01/16/2024 2:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC (UA)	0-2	0 - 2 / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
RBC (UA)	0-2	0 - 2 / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Epithelial Cells, Squamous (UA)	Few	None / LPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
Bacteria (UA)	Few	None / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
Amorphous Urates Crystals (UA)	Few	None / LPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
Budding Yeast (UA)	Few	None / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/16/2024 2:33 PM PST	01/16/2024 3:28 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS, MACROSCOPIC, W/REFLEX TO CULTURE (URINALYSIS W/REFLEX TO CULTURE) - Final result (01/16/2024 2:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Color (UA)	Yellow	Yellow		01/16/2024 3:43 PM PST	MERCY LAB CV	
Clarity (UA)	Clear	Clear		01/16/2024 3:43 PM PST	MERCY LAB CV	
Specific Gravity (UA)	1.025	1.005 - 1.030		01/16/2024 3:43 PM PST	MERCY LAB CV	
pH (UA)	6.0	5.0 - 8.5 pH		01/16/2024 3:43 PM PST	MERCY LAB CV	
Protein (UA)	30	Negative mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	
Glucose (UA)	Negative	Negative mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	
Ketones (UA)	Negative	Negative mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	
Bilirubin (UA)	Negative	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	
Occult Blood (UA)	Negative	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	
Leukocyte Esterase (UA)	Negative	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Nitrite (UA)	Positive	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	
Urobilinogen (UA)	0.2	<2.0 mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/16/2024 2:33 PM PST	01/16/2024 3:28 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINE CULTURE - Final result (01/16/2024 2:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Urine Culture	Multiple organisms present, probable skin/ urogenital contamination.		MIC	01/18/2024 11:59 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/16/2024 2:33 PM PST	01/16/2024 3:42 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (01/16/2024 11:25 AM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/16/2024 11:52 AM PST	

Impressions

01/16/2024 11:52 AM PST
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Narrative

01/16/2024 11:52 AM PST
EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/16/2024 11:25 AM

HISTORY: RULE OUT DVT

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

Procedure Note

Harold Pimenta, MD - 01/16/2024
Formatting of this note might be different from the original.
EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/16/2024 11:25 AM

HISTORY: RULE OUT DVT

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

IMPRESSION:
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	IMG US PROCEDURES	Final Result

CT chest without contrast (CT CHEST WITHOUT CONTRAST) - Final result (01/16/2024 9:51 AM PST)

Anatomical Region	Laterality	Modality
Chest		Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/16/2024 9:57 AM PST	

Impressions

01/16/2024 10:07 AM PST

IMPRESSION:

1. Small to moderate right pleural effusion with indwelling pleural drainage catheter. Subjacent right basilar atelectasis/consolidation

2. Right upper lobe groundglass opacity with peripheral consolidation (reverse halo sign). Findings can be seen in the setting of atypical community-acquired bacterial pneumonia, fungal pneumonia, and organizing pneumonia.

release

Narrative

01/16/2024 10:07 AM PST

EXAMINATION: CT CHEST WITHOUT CONTRAST

DATE/TIME: 1/16/2024 9:51 AM

HISTORY: 59 years old Female, Pneumonia

COMPARISON: Same day chest radiograph.

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

Lungs: Small to moderate right pleural effusion with indwelling pleural drainage catheter and scattered expected foci of pleural air. Subjacent right basilar atelectasis/consolidation. Right upper lobe groundglass opacity with mild peripheral consolidation (series 5/102). Multifocal right middle and lower lobe scarring and volume loss.

Airways: Patent trachea and central airways. Mild diffuse bronchial wall thickening.

Vasculature: Thoracic aortic atherosclerosis without aneurysm. Main pulmonary artery enlargement up to 3.8 cm.

Cardiac: Mild coronary artery calcifications. Mild cardiomegaly. No pericardial effusion.

Lymph nodes: Limited evaluation without IV contrast. No bulky supraclavicular, axillary, mediastinal, or hilar lymphadenopathy.

Other mediastinal structures: Mild esophageal wall thickening, which may be due to underdistention or esophagitis.

Chest wall: No significant abnormality.

Bones: Old healed midsternal fracture. Degenerative changes of the spine. No aggressive osseous lesion.

Upper abdomen: No significant abnormality.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: Chest; Phantom: 32 cm; CTDIvol: 5; DLP: 177

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

Procedure Note

Pranay Kothari, MD - 01/16/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: CT CHEST WITHOUT CONTRAST

DATE/TIME: 1/16/2024 9:51 AM

HISTORY: 59 years old Female, Pneumonia

COMPARISON: Same day chest radiograph.

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

Lungs: Small to moderate right pleural effusion with indwelling pleural drainage catheter and scattered expected foci of pleural air. Subjacent right basilar atelectasis/consolidation. Right upper lobe groundglass opacity with mild peripheral consolidation (series 5/102). Multifocal right middle and lower lobe scarring and volume loss.

Airways: Patent trachea and central airways. Mild diffuse bronchial wall thickening.

Vasculature: Thoracic aortic atherosclerosis without aneurysm. Main pulmonary artery enlargement up to 3.8 cm.

Cardiac: Mild coronary artery calcifications. Mild cardiomegaly. No pericardial effusion.

Lymph nodes: Limited evaluation without IV contrast. No bulky supraclavicular, axillary, mediastinal, or hilar lymphadenopathy.

Other mediastinal structures: Mild esophageal wall thickening, which may be due to underdistention or esophagitis.

Chest wall: No significant abnormality.

Bones: Old healed midsternal fracture. Degenerative changes of the spine. No aggressive osseous lesion.

Upper abdomen: No significant abnormality.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: Chest; Phantom: 32 cm; CTDIvol: 5; DLP: 177

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

IMPRESSION:
IMPRESSION:

1. Small to moderate right pleural effusion with indwelling pleural drainage catheter. Subjacent right basilar atelectasis/ consolidation

2. Right upper lobe groundglass opacity with peripheral consolidation (reverse halo sign). Findings can be seen in the setting of atypical community-acquired bacterial pneumonia, fungal pneumonia, and organizing pneumonia.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG CT PROCEDURES	Final Result

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/16/2024 6:27 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/16/2024 9:35 AM PST	

Impressions

01/16/2024 9:35 AM PST
IMPRESSION: No significant interval change.

release
CC Dr: p: f:

Narrative

01/16/2024 9:35 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/16/2024 6:27 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/15/2024

FINDINGS: Stable position of the right chest tube. Moderate volume right pleural effusion/thickening is seen with mild to moderate patchy and streak-like change of the right lung base. Stable cardiomegaly with mild bronchovascular prominence. No left lung infiltrate or clear-cut pneumothorax.

Procedure Note

Ross Christensen, MD - 01/16/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/16/2024 6:27 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/15/2024

FINDINGS: Stable position of the right chest tube. Moderate volume right pleural effusion/thickening is seen with mild to moderate patchy and streak-like change of the right lung base. Stable cardiomegaly with mild bronchovascular prominence. No left lung infiltrate or clear-cut pneumothorax.

IMPRESSION:
IMPRESSION: No significant interval change.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/16/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	13.1	3.4 - 11.0 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
RBC	3.37	3.98 - 5.25 M/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HGB	10.6	11.9 - 15.3 g/dL		01/16/2024 5:49 AM PST	MERCY LAB CV	
HCT	35.1	37.3 - 46.7 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
MCV	104	81 - 100 fL		01/16/2024 5:49 AM PST	MERCY LAB CV	
MCH	32	26 - 33 pg		01/16/2024 5:49 AM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		01/16/2024 5:49 AM PST	MERCY LAB CV	
RDW	14.5	11.7 - 14.9 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Platelet Count	546	150 - 425 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
MPV	9.3	9.0 - 12.8 fL		01/16/2024 5:49 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/16/2024 5:49 AM PST	MERCY LAB CV	
Neutrophils %	85.4	38.0 - 74.0 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
IG%	2.0	0.0 - 1.2 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Lymphocytes %	8.1	16.0 - 48.0 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Monocytes %	4.1	4.9 - 12.5 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Eosinophils %	0.2	0.0 - 9.5 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Neutrophils	11.21	1.50 - 7.40 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
IG#	0.26	0.00 - 0.10 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.07	0.90 - 3.10 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Monocytes	0.54	0.26 - 0.87 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.02	0.00 - 0.51 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Basophils	0.03	0.00 - 0.09 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Preliminary Abs Neut	11.21	1.50 - 7.40 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/16/2024 5:19 AM PST	01/16/2024 5:40 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/16/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/16/2024 9:33 AM PST	MERCY LAB CV	
Glucose	116	70 - 125 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/16/2024 9:33 AM PST	MERCY LAB CV	
Anion Gap	-6	6 - 14 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Osmolality Calculated	276	280 - 305 mOsm/kg H20		01/16/2024 9:33 AM PST	MERCY LAB CV	
Calcium	8.5	8.4 - 10.2 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protein, total	6.9	6.3 - 8.2 g/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Alkaline Phosphatase	95	38 - 126 Units/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
AST	102	14 - 36 Units/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Bilirubin, Total	0.4	0.2 - 1.3 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/16/2024 5:19 AM PST	01/16/2024 6:01 AM PST

Narrative

MERCY LAB CV - 01/16/2024 9:33 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/15/2024 11:42 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/15/2024 11:43 AM PST	

Impressions

01/15/2024 11:44 AM PST
IMPRESSION:
1. Worsening right basilar atelectasis/consolidation.
2. Persistent right pleural effusion.
3. Right chest tube is unchanged. No pneumothorax.

release
CC Dr: p: f:

Narrative

01/15/2024 11:44 AM PST

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/15/2024 11:42 AM

HISTORY: Respiratory failure

COMPARISON: 1/14/2024

FINDINGS:
Endotracheal tube has been removed.

Right chest tube is unchanged.

There is no significant pneumothorax.

Increase in right basilar hazy and linear pulmonary opacification. Right costophrenic angle remains blunted.

Left costophrenic angle is sharp.

There is no evidence of pneumothorax.

Cardiomegaly.

Procedure Note

Stephen Hulse, MD - 01/15/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/15/2024 11:42 AM

HISTORY: Respiratory failure

COMPARISON: 1/14/2024

FINDINGS:
Endotracheal tube has been removed.

Right chest tube is unchanged.

There is no significant pneumothorax.

Increase in right basilar hazy and linear pulmonary opacification. Right costophrenic angle remains blunted.

Left costophrenic angle is sharp.

There is no evidence of pneumothorax.

Cardiomegaly.

IMPRESSION:
IMPRESSION:
1. Worsening right basilar atelectasis/consolidation.
2. Persistent right pleural effusion.
3. Right chest tube is unchanged. No pneumothorax.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

GLUCOSE, POC - Final result (01/15/2024 5:52 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	97	70 - 125 mg/dL		01/15/2024 5:54 AM PST	MERCY LAB CV	
Operator ID	233,925			01/15/2024 5:54 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/15/2024 5:52 AM PST	01/15/2024 5:54 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/15/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	12.5	3.4 - 11.0 K/mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
RBC	3.31	3.98 - 5.25 M/mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
HGB	10.4	11.9 - 15.3 g/dL		01/15/2024 5:47 AM PST	MERCY LAB CV	
HCT	33.9	37.3 - 46.7 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/15/2024 5:47 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/15/2024 5:47 AM PST	MERCY LAB CV	
MCHC	30.7	31.0 - 36.0 g/dL		01/15/2024 5:47 AM PST	MERCY LAB CV	
RDW	14.6	11.7 - 14.9 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Platelet Count	543	150 - 425 K/mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
MPV	9.3	9.0 - 12.8 fL		01/15/2024 5:47 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/15/2024 5:47 AM PST	MERCY LAB CV	
Neutrophils %	85.7	38.0 - 74.0 %		01/15/2024 5:47 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
IG%	2.6	0.0 - 1.2 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Lymphocytes %	7.5	16.0 - 48.0 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Monocytes %	3.8	4.9 - 12.5 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Eosinophils %	0.2	0.0 - 9.5 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Neutrophils	10.68	1.50 - 7.40 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
IG#	0.33	0.00 - 0.10 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Lymphocytes	0.93	0.90 - 3.10 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Monocytes	0.48	0.26 - 0.87 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.02	0.00 - 0.51 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Preliminary Abs Neut	10.68	1.50 - 7.40 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/15/2024 5:28 AM PST	01/15/2024 5:43 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (01/15/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/15/2024 6:12 AM PST	MERCY LAB CV	
Glucose	102	70 - 125 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	135	137 - 145 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
CO2, Total	34	22 - 30 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Urea Nitrogen	33	7 - 17 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	
Creatinine	0.90	0.50 - 1.00 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	74	>60 mL/min/1.73m*2		01/15/2024 6:12 AM PST	MERCY LAB CV	
Anion Gap	-1	6 - 14 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Osmolality Calculated	287	280 - 305 mOsm/kg H20		01/15/2024 6:12 AM PST	MERCY LAB CV	
Calcium	8.4	8.4 - 10.2 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/15/2024 5:28 AM PST	01/15/2024 5:44 AM PST

Narrative

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MERCY LAB CV - 01/15/2024 6:12 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/15/2024 12:15 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	103	70 - 125 mg/dL		01/15/2024 12:19 AM PST	MERCY LAB CV	
Operator ID	233,925			01/15/2024 12:19 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/15/2024 12:15 AM PST	01/15/2024 12:19 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Truc Ho MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 5:17 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	93	70 - 125 mg/dL		01/14/2024 5:19 PM PST	MERCY LAB CV	
Operator ID	244,516			01/14/2024 5:19 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 5:17 PM PST	01/14/2024 5:19 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 12:29 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	101	70 - 125 mg/dL		01/14/2024 12:32 PM PST	MERCY LAB CV	
Operator ID	145,602			01/14/2024 12:32 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 12:29 PM PST	01/14/2024 12:32 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 6:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	94	70 - 125 mg/dL		01/14/2024 6:19 AM PST	MERCY LAB CV	
Operator ID	232,690			01/14/2024 6:19 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 6:11 AM PST	01/14/2024 6:19 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	14.3	3.4 - 11.0 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
RBC	3.14	3.98 - 5.25 M/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
HGB	9.7	11.9 - 15.3 g/dL		01/14/2024 5:52 AM PST	MERCY LAB CV	
HCT	30.4	37.3 - 46.7 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
MCV	97	81 - 100 fL		01/14/2024 5:52 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/14/2024 5:52 AM PST	MERCY LAB CV	
MCHC	31.9	31.0 - 36.0 g/dL		01/14/2024 5:52 AM PST	MERCY LAB CV	
RDW	14.6	11.7 - 14.9 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Platelet Count	573	150 - 425 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
MPV	9.6	9.0 - 12.8 fL		01/14/2024 5:52 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
nRBC	0.0	0.0 /100 WBCs		01/14/2024 5:52 AM PST	MERCY LAB CV	
Neutrophils %	84.2	38.0 - 74.0 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
IG%	2.8	0.0 - 1.2 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Lymphocytes %	9.7	16.0 - 48.0 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Monocytes %	3.0	4.9 - 12.5 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Eosinophils %	0.1	0.0 - 9.5 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Neutrophils	12.01	1.50 - 7.40 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
IG#	0.40	0.00 - 0.10 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.39	0.90 - 3.10 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Monocytes	0.43	0.26 - 0.87 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.01	0.00 - 0.51 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Preliminary Abs Neut	12.01	1.50 - 7.40 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:22 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MAGNESIUM - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Magnesium	2.3	1.6 - 2.3 mg/dL		01/14/2024 5:45 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

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MERCY LAB CV - 01/14/2024 5:45 AM PST

NOTE: Higher values can be expected in females during menses.
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Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CK - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
CK	128	30 - 135 Units/L		01/14/2024 5:45 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

VANCOMYCIN LEVEL, RANDOM - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Vancomycin Level, Random	16	Not Established mcg/mL		01/14/2024 5:50 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/14/2024 7:41 AM PST	MERCY LAB CV	
Glucose	144	70 - 125 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	
Sodium	133	137 - 145 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Chloride	104	98 - 107 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
CO2, Total	26	22 - 30 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Urea Nitrogen	50	7 - 17 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	
Creatinine	1.20	0.50 - 1.00 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	52	>60 mL/min/1.73m*2		01/14/2024 7:41 AM PST	MERCY LAB CV	
Anion Gap	3	6 - 14 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Osmolality Calculated	292	280 - 305 mOsm/kg H20		01/14/2024 7:41 AM PST	MERCY LAB CV	
Calcium	8.1	8.4 - 10.2 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

MERCY LAB CV - 01/14/2024 7:41 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/14/2024 4:29 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/14/2024 9:20 AM PST	

Impressions

01/14/2024 9:22 AM PST
IMPRESSION:
Essentially unchanged examination.

release

Narrative

01/14/2024 9:22 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/14/2024 4:29 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/13/2024

FINDINGS:
One view obtained.

Right basilar atelectasis/consolidation. There is a small right pleural effusion. Right chest tube remains in place. No appreciable pneumothorax. Left lung appears clear.

The cardiomediastinal silhouette is enlarged. Tip of the endotracheal tube projects 4.1 cm superior to the carina. Enteric tube passes below the diaphragm, tip not seen. The osseous structures demonstrate no acute abnormality.

Procedure Note

Anshuman Bansal, MD - 01/14/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/14/2024 4:29 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/13/2024

FINDINGS:
One view obtained.

Right basilar atelectasis/consolidation. There is a small right pleural effusion. Right chest tube remains in place. No appreciable pneumothorax. Left lung appears clear.

The cardiomediastinal silhouette is enlarged. Tip of the endotracheal tube projects 4.1 cm superior to the carina. Enteric tube passes below the diaphragm, tip not seen. The osseous structures demonstrate no acute abnormality.

IMPRESSION:
IMPRESSION:
Essentially unchanged examination.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

BLOOD GAS LACTIC ACID, ARTERIAL - Final result (01/14/2024 3:47 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.45	7.35 - 7.45		01/14/2024 4:06 AM PST	MERCY LAB CV	
pCO2, Arterial	42	35 - 45 mm Hg		01/14/2024 4:06 AM PST	MERCY LAB CV	
pO2, Arterial	56	83 - 108 mm Hg		01/14/2024 4:06 AM PST	MERCY LAB CV	
HCO3, Arterial	29.4	21.0 - 28.0 mmol/L		01/14/2024 4:06 AM PST	MERCY LAB CV	
O2 Sat, Arterial	90.1	95.0 - 98.0 %		01/14/2024 4:06 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	5.5	-2.0 - 3.0 mmol/L		01/14/2024 4:06 AM PST	MERCY LAB CV	
FIO2, Arterial	35.0			01/14/2024 4:06 AM PST	MERCY LAB CV	
Oxygen Device	Ventilator			01/14/2024 4:06 AM PST	MERCY LAB CV	
Modes	PRVC			01/14/2024 4:06 AM PST	MERCY LAB CV	
Lactic Acid, Arterial	1.0	0.4 - 0.9 mmol/L		01/14/2024 4:06 AM PST	MERCY LAB CV	
Allen test	NA			01/14/2024 4:06 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/14/2024 4:06 AM PST	MERCY LAB CV	
Comment	400/20/5			01/14/2024 4:06 AM PST	MERCY LAB CV	
Notified By	243560			01/14/2024 4:06 AM PST	MERCY LAB CV	
Notified Whom	258646			01/14/2024 4:06 AM PST	MERCY LAB CV	
Operator ID	243,560			01/14/2024 4:06 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/14/2024 3:47 AM PST	01/14/2024 4:06 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/14/2024 12:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	173	70 - 125 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Sodium	134	137 - 145 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Chloride	104	98 - 107 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
CO2, Total	28	22 - 30 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Urea Nitrogen	53	7 - 17 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Creatinine	1.22	0.50 - 1.00 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	51	>60 mL/min/1.73m*2		01/14/2024 12:41 AM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Osmolality Calculated	297	280 - 305 mOsm/kg H20		01/14/2024 12:41 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Phosphorus	2.9	2.5 - 4.5 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 12:22 AM PST	01/14/2024 12:26 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 12:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	94	70 - 125 mg/dL		01/14/2024 12:21 AM PST	MERCY LAB CV	
Operator ID	234,267			01/14/2024 12:21 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 12:19 AM PST	01/14/2024 12:21 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

STREP PNEUMO ANTIGEN, URINE - Final result (01/13/2024 11:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Strep Pneumo Ag,UR	Positive	Negative		01/14/2024 3:48 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown	Non-Lab Collection / Unknown	01/13/2024 11:41 PM PST	01/13/2024 11:58 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

LEGIONELLA ANTIGEN URINE (LEGIONELLA ANTIGEN, URINE) - Final result (01/13/2024 11:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Legionella Ag, Urine	Negative	Negative		01/14/2024 3:48 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown	Non-Lab Collection / Unknown	01/13/2024 11:41 PM PST	01/13/2024 11:58 PM PST

Narrative

SCRIPPS LAB MICROBIOLOGY - 01/14/2024 3:48 PM PST
Negative (presumptive) for Legionella pneumophilia, serogroup 1 antigen. Suggests no recent or current infection. Infection cannot be ruled out as antigen may not be present in urine in early infection or the level of antigen may be below detectable limits.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

CULTURE, RESPIRATORY (RESPIRATORY CULTURE WITH STAIN) - Final result (01/13/2024 9:40 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Culture	No Growth Final		MIC	01/16/2024 8:09 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	Moderate WBCs			01/16/2024 8:09 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	No organisms seen			01/16/2024 8:09 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Sputum	Sputum / Unknown	Non-Lab Collection / Unknown	01/13/2024 9:40 PM PST	01/13/2024 9:43 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/13/2024 8:28 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	135	70 - 125 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Potassium	4.7	3.5 - 5.1 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
CO2, Total	27	22 - 30 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Urea Nitrogen	56	7 - 17 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Creatinine	1.30	0.50 - 1.00 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	47	>60 mL/min/1.73m*2		01/13/2024 8:50 PM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Osmolality Calculated	292	280 - 305 mOsm/kg H20		01/13/2024 8:50 PM PST	MERCY LAB CV	
Calcium	8.0	8.4 - 10.2 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Phosphorus	3.5	2.5 - 4.5 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 8:28 PM PST	01/13/2024 8:31 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/13/2024 6:07 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	99	70 - 125 mg/dL		01/13/2024 6:09 PM PST	MERCY LAB CV	
Operator ID	155,359			01/13/2024 6:09 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 6:07 PM PST	01/13/2024 6:09 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BETA HYDROXYBUTYRATE - Final result (01/13/2024 3:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Beta-Hydroxybutyrate	2.2	0.2 - 2.8 mg/dL		01/13/2024 3:59 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 3:33 PM PST	01/13/2024 3:36 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

VANCOMYCIN LEVEL, RANDOM - Final result (01/13/2024 3:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Vancomycin Level, Random	14	Not Established mcg/mL		01/13/2024 3:56 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 3:33 PM PST	01/13/2024 3:36 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (01/13/2024 3:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/13/2024 3:51 PM PST	MERCY LAB CV	
Glucose	132	70 - 125 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	131	137 - 145 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Potassium	5.3	3.5 - 5.1 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
CO2, Total	25	22 - 30 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Urea Nitrogen	59	7 - 17 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	
Creatinine	1.20	0.50 - 1.00 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	52	>60 mL/min/1.73m*2		01/13/2024 3:51 PM PST	MERCY LAB CV	
Anion Gap	6	6 - 14 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Osmolality Calculated	290	280 - 305 mOsm/kg H20		01/13/2024 3:51 PM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 3:33 PM PST	01/13/2024 3:36 PM PST

Narrative

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MERCY LAB CV - 01/13/2024 3:51 PM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

Ultrasound abdominal limited liver (ULTRASOUND ABDOMEN LIMITED LIVER) - Final result (01/13/2024 2:52 PM PST)

Anatomical Region	Laterality	Modality
Body		Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 2:54 PM PST	

Impressions

01/13/2024 2:55 PM PST

IMPRESSION: Unremarkable gallbladder.

Echogenic liver. Consider steatosis or hepatocellular disease.

Right pleural effusion

release

Narrative

01/13/2024 2:55 PM PST

EXAMINATION: ULTRASOUND ABDOMEN LIMITED LIVER

DATE/TIME: 1/13/2024 2:52 PM

HISTORY: Reported abdominal pain, please also evaluate gallbladder

COMPARISON: None.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: Length is 19.1 cm. Increased echogenicity. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
BILIARY AND GALLBLADDER: Normal gallbladder. No ductal dilatation. CBD diameter is 5.1 mm.
PANCREAS: Not well visualized
RIGHT KIDNEY: Right renal length is 10.0 cm. No visible solid mass, calculus or hydronephrosis.
AORTA/IVC/VASC: No visible aneurysm. The intrahepatic IVC is patent
OTHER: Right pleural effusion

Procedure Note

Raymond Hong, MD - 01/13/2024

Formatting of this note might be different from the original.

EXAMINATION: ULTRASOUND ABDOMEN LIMITED LIVER

DATE/TIME: 1/13/2024 2:52 PM

HISTORY: Reported abdominal pain, please also evaluate gallbladder

COMPARISON: None.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: Length is 19.1 cm. Increased echogenicity. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
BILIARY AND GALLBLADDER: Normal gallbladder. No ductal dilatation. CBD diameter is 5.1 mm.
PANCREAS: Not well visualized
RIGHT KIDNEY: Right renal length is 10.0 cm. No visible solid mass, calculus or hydronephrosis.
AORTA/IVC/VASC: No visible aneurysm. The intrahepatic IVC is patent
OTHER: Right pleural effusion

IMPRESSION:
IMPRESSION: Unremarkable gallbladder.

Echogenic liver. Consider steatosis or hepatocellular disease.

Right pleural effusion

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG US PROCEDURES	Final Result

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/13/2024 12:38 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	347	70 - 125 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Potassium	5.8	3.5 - 5.1 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
CO2, Total	20	22 - 30 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Urea Nitrogen	55	7 - 17 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Creatinine	1.10	0.50 - 1.00 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	58	>60 mL/min/1.73m*2		01/13/2024 1:14 PM PST	MERCY LAB CV	
Anion Gap	9	6 - 14 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Osmolality Calculated	301	280 - 305 mOsm/kg H20		01/13/2024 1:14 PM PST	MERCY LAB CV	
Calcium	7.9	8.4 - 10.2 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Phosphorus	5.7	2.5 - 4.5 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 12:38 PM PST	01/13/2024 12:48 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/13/2024 12:09 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 12:16 PM PST	

Impressions

01/13/2024 12:17 PM PST
IMPRESSION: Interval decrease in moderate right pleural effusion status post chest tube insertion. No pneumothorax.

release

Narrative

01/13/2024 12:17 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 12:09 PM

HISTORY: s/p chest tube insertion

COMPARISON: 1/13/2024

FINDINGS:

New right chest tube. Endotracheal and gastric tubes are in stable position.

The heart and mediastinum are unchanged.

Interval decrease in moderate right pleural effusion. No pneumothorax. Right basilar atelectasis or infiltrate again noted.

Imaged skeletal structures are unchanged.

Procedure Note

Raymond Hong, MD - 01/13/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 12:09 PM

HISTORY: s/p chest tube insertion

COMPARISON: 1/13/2024

FINDINGS:

New right chest tube. Endotracheal and gastric tubes are in stable position.

The heart and mediastinum are unchanged.

Interval decrease in moderate right pleural effusion. No pneumothorax. Right basilar atelectasis or infiltrate again noted.

Imaged skeletal structures are unchanged.

IMPRESSION:
IMPRESSION: Interval decrease in moderate right pleural effusion status post chest tube insertion. No pneumothorax.

release

Authorizing Provider	Result Type	Result Status
Brandon May DNP	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

GLUCOSE, POC - Final result (01/13/2024 12:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	111	70 - 125 mg/dL		01/13/2024 4:12 PM PST	MERCY LAB CV	
Operator ID	244,516			01/13/2024 4:12 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 12:03 PM PST	01/13/2024 4:12 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CHEST TUBE INSERTION (01/13/2024 11:44 AM PST)

Narrative

Pedro Dammert Coronado, MD - 01/13/2024 11:44 AM PST

Narrative

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May, Brandon Michael, DNP     1/13/2024 11:46 AM
Chest Tube Insertion

Date/Time: 1/13/2024 11:44 AM

Performed by: May, Brandon Michael, DNP
Authorized by: May, Brandon Michael, DNP
Consent:
 Consent obtained:  Written
 Consent given by:  Healthcare agent
 Risks, benefits, and alternatives were discussed: yes
 Risks discussed:  Bleeding, damage to surrounding structures, nerve
damage, pain, infection and incomplete drainage
 Alternatives discussed:  No treatment
Universal protocol:
 Procedure explained and questions answered to patient or proxy's
satisfaction: yes
 Relevant documents present and verified: yes
 Test results available: yes
 Imaging studies available: yes
 Required blood products, implants, devices, and special equipment
available: yes
 Site/side marked: yes
 Immediately prior to procedure, a time out was called: yes
 Patient identity confirmed:  Hospital-assigned identification number and
arm band
Pre-procedure details:
 Skin preparation:  Chlorhexidine with alcohol
 Preparation: Patient was prepped and draped in the usual sterile fashion

Anesthesia:
 Anesthesia method:  Local infiltration
 Local anesthetic:  Lidocaine 1% w/o epi
Procedure details:
 Placement location:  R lateral
 Tube size (French): 14.
 Ultrasound guidance: yes
 Tension pneumothorax: no
 Tube connected to:  Water seal
 Drainage characteristics:  Purulent and yellow
 Suture material:  2-0 silk
 Dressing: Tegaderm.
Post-procedure details:
 Procedure completion:  Tolerated well, no immediate complications
Comments:
  CXR pending, Dr. Dammert at bedside for the entire procedure
```

Authorizing Provider	Result Type	Result Status
Brandon May DNP	PROCEDURE/MINOR SURGICAL ORDERABLES	Final Result

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (01/13/2024 10:57 AM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 11:27 AM PST	

Impressions

```
01/13/2024 11:27 AM PST
IMPRESSION:  Negative for deep vein thrombosis in both lower extremities.

1.7 cm right popliteal cyst


release
```

Narrative

01/13/2024 11:27 AM PST

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/13/2024 10:57 AM

HISTORY: Asymmetric edema R>L

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: 1.7 cm right popliteal cyst.

Procedure Note

Raymond Hong, MD - 01/13/2024

Formatting of this note might be different from the original.

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/13/2024 10:57 AM

HISTORY: Asymmetric edema R>L

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: 1.7 cm right popliteal cyst.

IMPRESSION:
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

1.7 cm right popliteal cyst

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG US PROCEDURES	Final Result

2D ECHO COMPLETE WITHOUT CONTRAST (WITH COLOR AND SPECTRAL DOPPLER AND MYOCARDIAL STRAIN) (01/13/2024 9:44 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Interventricular Septal Thickness End Diastole	1.1	cm			SCRIPPS RADIOLOGY	
Left Ventricular Internal Diameter End Diastole	6.2	cm			SCRIPPS RADIOLOGY	
Left Ventricular Internal Diameter End Systole	5.4	cm			SCRIPPS RADIOLOGY	
Left Ventricular Posterior Wall	1.1	cm			SCRIPPS RADIOLOGY	
Left Ventricular End Systolic Volume Indexed	94	ml			SCRIPPS RADIOLOGY	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Left Ventricular Ejection Fraction	26.2	%			SCRIPPS RADIOLOGY	
Mitral Valve E/A Ratio	0.89				SCRIPPS RADIOLOGY	
Aortic Valve Maximum Velocity	146	cm/sec			SCRIPPS RADIOLOGY	
Aortic Valve Mean Pressure Gradient	4	mmHg			SCRIPPS RADIOLOGY	
Aortic Valve Area	2	cm^2			SCRIPPS RADIOLOGY	
LVOT VTI	18.2	cm			SCRIPPS RADIOLOGY	
Mitral Regurgitation Effective Regurgitant Orifice Area	0.13	cm^2			SCRIPPS RADIOLOGY	
Mitral Regurgitant Volume	20.9	ml			SCRIPPS RADIOLOGY	
Stroke Volume (LVOT) Indexed	26.7	ml/m^2			SCRIPPS RADIOLOGY	
RA Volume (Indexed)	36.3	ml/m^2			SCRIPPS RADIOLOGY	
Mitral Valve Average E/e' Ratio	13.2				SCRIPPS RADIOLOGY	
Left Ventricular End Diastolic Volume Indexed	73.3	ml/m^2			SCRIPPS RADIOLOGY	
Left Atrial Volume Indexed	38.3				SCRIPPS RADIOLOGY	
Ascending Aorta Indexed	2.1	cm			SCRIPPS RADIOLOGY	
Ao Sinus Indexed	2.2	cm			SCRIPPS RADIOLOGY	
Right Ventricular Diameter 2 (mid-level)	1.6	cm			SCRIPPS RADIOLOGY	
TAPSE	1.6	cm			SCRIPPS RADIOLOGY	
Dimensionless Index (LVOT/AoV)	0.81				SCRIPPS RADIOLOGY	

Anatomical Region	Laterality	Modality
		Echocardiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 8:39 AM PST	

Impressions

01/14/2024 7:37 AM PST

Study ID: 1074047

Scripps Mercy Hospital Chula Vista
Cardiology Lab
435 H Street
Chula Vista, CA 91910
(619) 691-7499

Adult Echo Report

Name: PETRUCCIO, ELIZABETH TINA
MRN: 810760324
DOB: 07/29/1964
Age: 59 yrs
Gender: Female
HR: 66
Study Date: 01/13/2024 08:39 AM
Patient Location: SVICU^OICU^11^CV
Height: 67 in
Weight: 140 lb
BSA: 1.7 m2
BP: 105/83 mmHg

Reason For Study: Other Cardiac Reason, Specify in Comments
History: Heart failure reduced ejection fraction on echo, reported history of asthma; medical noncompliance, past alcohol and methamphetamine abuse
Acct Number: 10143864623
Accession Number: 3011620730

Impressions

EXAM/PROCEDURE
Complete 2D/M-mode, color Doppler, and spectral Doppler echocardiogram performed. The study was technically adequate. No prior exam for comparison. Critical value: new LV systolic dysfunction, EF <35%. Sonographer communicated preliminary findings to Dr Damert Coronado at 09:44 on 01/13/2024.

Interpretation Summary
The left ventricle is severely dilated.
Global left ventricular systolic function is severely reduced.

LEFT VENTRICLE
The left ventricle is severely dilated.LVIDd = 6.2 cm. There is increased left ventricular mass with normal relative wall thickness consistent with severe eccentric hypertrophy. LV mass index = 168.0 grams/m^2. Relative wall thickness= 0.36 (nl <0.42).The ejection fraction = 26.2 % by biplane Method of Discs. Ejection Fraction = 25.5 %. Global longitudinal strain is -6.9%. Global left ventricular systolic function is severely reduced. There is no thrombus visualized. A false chord is noted (normal variant).

DIASTOLOGY/HEMODYNAMICS
There is Stage I impaired LV diastolic filling pattern. Avg. E/e' = 13.2 (nl <13). There is evidence for elevated left atrial pressure. Cardiac output is 3.4 l/min (nl 4-7l/min). Cardiac index is 1.9 l/min/m^2 (nl 2.5-4.5l/min/m2). Unable to estimate pulmonary arterial
pressure. Right atrial pressure is estimated as 15mmHg.

RIGHT VENTRICLE
The right ventricle is dilated. RVDd2 = 1.6 cm. RVDd1 = 4.6 cm. The right ventricular ejection fraction is mildly decreased. The right ventricular tricuspid annular plane systolic excursion (TAPSE) measures 1.6 cm.

ATRIA
The left atrium is mildly dilated. Left atrial volume (bi-plane) = 38.3 ml/m2 (nl < 35ml/m2). Right atrium is dilated. Right atrial volume indexed = 36.3 ml/m^2 (nl male/female <39/<34 ml/m^2). There is no Doppler evidence for a patent foramen ovale.

MITRAL VALVE
The mitral regurgitant jet is eccentrically directed. There is mild to moderate mitral regurgitation. Effective regurgitant orifice 0.13 cm^2 (severe >0.4cm2). Mitral regurgitation PISA = 0.50 cm. Regurgitant volume 20.9 ml (severe >/= 60ml/beat).

AORTIC VALVE
Normal trileaflet aortic valve. Trace aortic regurgitation.

TRICUSPID VALVE
The tricuspid valve is normal. There is physiologic tricuspid regurgitation.

PULMONIC VALVE
The pulmonic valve leaflets are thin and pliable; valve motion is normal. Trace pulmonic valvular regurgitation.

AORTA/PULMONARY ARTERY
The aortic sinus of valsalva is dilated. The Aortic Sinus(es) of Valsalva measures 3.8 cm. Indexed Aortic Sinus(es) of Valsalva are 2.2 cm/m2. (normal <=2.1cm/m2). The ascending aorta is dilated. The ascending aorta measures 3.7 cm. Indexed ascending aorta is 2.1 cm/m2. (normal <=1.9cm/m2). The pulmonary artery is dilated. The pulmonary artery measures 3.9 cm.

PERICARDIUM/PLEURAL
There is no pericardial effusion. Large right pleural effusion. The left lung base image was technically limited. A unilateral comet-tail artifact is visualized when imaging the anteroapex of the lung.

MMode/2D Measurements & Calculations

RVDd1(nl <4.2cm): 4.6 cm	LVIDd (3.8-5.8): 6.2 cm
RVDd2(nl <3.6cm): 1.6 cm	LVIDs (2.3-3.9): 5.4 cm
IVSd (0.6-1.1): 1.1 cm	LVPWd (0.6-1.1): 1.1 cm
LV mass(C)d: 291.9 grams	Ao sinus diam: 3.8 cm
LV mass idx (F<96,M<116g/m2): 168.0 grams/m2	AO sinus indexed: 2.2 cm
Relative Wall Thickness: 0.36	asc Aorta Diam: 3.7 cm
	asc AO indexed: 2.1 cm
	LA dimension (1.9-4.0): 3.9 cm
LVOT diam: 1.8 cm	
	LA area AP2: 26.1 cm2
LVOT area: 2.5 cm2	LA area AP4: 18.3 cm2
	LA shortest length: 6.1 cm
	LA VOLUME (indexed): 38.3
MPA diam: 3.9 cm	

Impressions

EDV(MOD-bp): 127.4 ml

EDV(MOD-bp) indexed: 73.3 ml/m2
ESV(MOD-bp): 94.0 ml
EF(MOD-bp): 26.2 %
SV(MOD-bp): 33.3 ml

RA area AP4: 17.7 cm2
RA length: 5.0 cm
RA Volume Indexed: 36.3 ml/m2
TAPSE(nl >=1.7cm): 1.6 cm

Time Measurements
Aortic R-R: 0.82 sec
Aortic HR: 73.0 BPM

Doppler Measurements & Calculations
MV E max vel: 46.0 cm/sec
MV A max vel: 51.8 cm/sec
MV E/A: 0.89
MV dec slope: 206.0 cm/sec2
MV dec time: 0.22 sec
Lat Peak E' Vel: 3.5 cm/sec
Med Peak E' Vel: 3.5 cm/sec
Lateral E/e' ratio: 13.2
Average E/e' ratio: 13.2

Ao V2 max: 146.0 cm/sec
Ao max PG: 8.5 mmHg
Ao V2 mean: 99.4 cm/sec
Ao mean PG: 4.0 mmHg
Ao V2 VTI: 22.6 cm
AVA(I,D): 2.0 cm2

AVA (I,D) indexed for BSA: 1.2 cm2
Dimensionless Index (LV/AV): 0.81
LV V1 max PG: 4.3 mmHg
LV V1 mean PG: 2.0 mmHg
LV V1 max: 104.0 cm/sec
LV V1 mean: 74.5 cm/sec
LV V1 VTI: 18.2 cm

MR max vel: 451.0 cm/sec
MR max PG: 81.4 mmHg
MR mean vel: 355.0 cm/sec
MR mean PG: 55.0 mmHg
MR VTI: 156.0 cm
MR PISA radius: 0.50 cm
MR alias vel: 38.5 cm/sec
MR ERO: 0.13 cm2
MR volume: 20.9 ml
MR flow rate: 60.5 cm3/sec
CO(LVOT): 3.4 l/min
CI(LVOT): 1.9 l/min/m2
SV(LVOT): 46.3 ml
SI(LVOT): 26.7 ml/m2

PA V2 max: 73.0 cm/sec
PA max PG: 2.0 mmHg
RV V1 max: 56.1 cm/sec
RV V1 max PG: 1.3 mmHg
RV S'(nl>=9.5cm/sec): 12.4 cm/sec

MR regurgitant fraction: 31.1 %

Reading Physician:Electronically signed by: Arvin Narula, MD on 01/14/2024 07:37 AM
Ordering Physician: JONES, CHRISTOPHER
Performed By: Rodil, Alla, RDCS

Narrative

This result has an attachment that is not available.

Procedure Note

Arvin Narula, MD - 01/14/2024

Formatting of this note might be different from the original.

IMPRESSION:

Study ID: 1074047

Scripps Mercy Hospital Chula Vista
Cardiology Lab
435 H Street
Chula Vista, CA 91910
(619) 691-7499
Adult Echo Report
Name: PETRUCCIO, ELIZABETH TINA Study Date: 01/13/2024 08:39 AM
MRN: 810760324 Patient Location: SVICU^0ICU^11^CV

Procedure Note

DOB: 07/29/1964 Height: 67 in
Age: 59 yrs Weight: 140 lb
Gender: Female BSA: 1.7 m2
HR: 66 BP: 105/83 mmHg
Reason For Study: Other Cardiac Reason, Specify in Comments
History: Heart failure reduced ejection fraction on echo, reported history of asthma; medical noncompliance, past alcohol and methamphetamine abuse
Acct Number: 10143864623 Accession Number: 3011620730

EXAM/PROCEDURE
Complete 2D/M-mode, color Doppler, and spectral Doppler echocardiogram performed. The study was technically adequate. No prior exam for comparison. Critical value: new LV systolic dysfunction, EF <35%. Sonographer communicated preliminary findings to Dr Damert Coronado at 09:44 on 01/13/2024.

Interpretation Summary
The left ventricle is severely dilated.
Global left ventricular systolic function is severely reduced.

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The left ventricle is severely dilated.LVIDd = 6.2 cm. There is increased left ventricular mass with normal relative wall thickness consistent with severe eccentric hypertrophy. LV mass index = 168.0 grams/m^2. Relative wall thickness= 0.36 (nl <0.42).The ejection fraction = 26.2 % by biplane Method of Discs. Ejection Fraction = 25.5 %. Global longitudinal strain is -6.9%. Global left ventricular systolic function is severely reduced. There is no thrombus visualized. A false chord is noted (normal variant).

DIASTOLOGY/HEMODYNAMICS
There is Stage I impaired LV diastolic filling pattern. Avg. E/e' = 13.2 (nl <13). There is evidence for elevated left atrial pressure. Cardiac output is 3.4 l/min (nl 4-7l/min). Cardiac index is 1.9 l/min/m^2 (nl 2.5-4.5l/min/m2). Unable to estimate pulmonary arterial
pressure. Right atrial pressure is estimated as 15mmHg.

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The right ventricle is dilated. RVDd2 = 1.6 cm. RVDd1 = 4.6 cm. The right ventricular ejection fraction is mildly decreased. The right ventricular tricuspid annular plane systolic excursion (TAPSE) measures 1.6 cm.

ATRIA
The left atrium is mildly dilated. Left atrial volume (bi-plane) = 38.3 ml/m2 (nl < 35ml/m2). Right atrium is dilated. Right atrial volume indexed = 36.3 ml/m^2 (nl male/female <39/<34 ml/m^2). There is no Doppler evidence for a patent foramen ovale.

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AORTIC VALVE
Normal trileaflet aortic valve. Trace aortic regurgitation.

TRICUSPID VALVE
The tricuspid valve is normal. There is physiologic tricuspid regurgitation.

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The pulmonic valve leaflets are thin and pliable; valve motion is normal. Trace pulmonic valvular regurgitation.

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The aortic sinus of valsalva is dilated. The Aortic Sinus(es) of Valsalva measures 3.8 cm. Indexed Aortic Sinus(es) of Valsalva are 2.2 cm/m2. (normal <=2.1cm/m2). The ascending aorta is dilated. The ascending aorta measures 3.7 cm. Indexed ascending aorta is 2.1 cm/m2. (normal <=1.9cm/m2). The pulmonary artery is dilated. The pulmonary artery measures 3.9 cm.

PERICARDIUM/PLEURAL
There is no pericardial effusion. Large right pleural effusion. The left lung base image

was technically limited. A unilateral comet-tail artifact is visualized when imaging the anteroapex of the lung.

MMode/2D Measurements & Calculations
RVDd1(nl <4.2cm): 4.6 cm LVIDd (3.8-5.8): 6.2 cm
RVDd2(nl <3.6cm): 1.6 cm LVIDs (2.3-3.9): 5.4 cm
IVSd (0.6-1.1): 1.1 cm LVPWd (0.6-1.1): 1.1 cm

LV mass(C)d: 291.9 grams Ao sinus diam: 3.8 cm
LV mass idx (F<96,M<116g/m2): 168.0 grams/m2 AO sinus indexed: 2.2 cm
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asc AO indexed: 2.1 cm
LA dimension (1.9-4.0): 3.9 cm

LVOT diam: 1.8 cm
LA area AP2: 26.1 cm2
LVOT area: 2.5 cm2 LA area AP4: 18.3 cm2
LA shortest length: 6.1 cm
LA VOLUME (indexed): 38.3

MPA diam: 3.9 cm
EDV(MOD-bp): 127.4 ml

EDV(MOD-bp) indexed: 73.3 ml/m2
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Aortic R-R: 0.82 sec
Aortic HR: 73.0 BPM

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MV A max vel: 51.8 cm/sec Med Peak E' Vel: 3.5 cm/sec
MV E/A: 0.89 Lateral E/e' ratio: 13.2
MV dec slope: 206.0 cm/sec2 Average E/e' ratio: 13.2
MV dec time: 0.22 sec

Ao V2 max: 146.0 cm/sec LV V1 max PG: 4.3 mmHg
Ao max PG: 8.5 mmHg LV V1 mean PG: 2.0 mmHg
Ao V2 mean: 99.4 cm/sec LV V1 max: 104.0 cm/sec
Ao mean PG: 4.0 mmHg LV V1 mean: 74.5 cm/sec
Ao V2 VTI: 22.6 cm LV V1 VTI: 18.2 cm
AVA(I,D): 2.0 cm2

AVA (I,D) indexed for BSA: 1.2 cm2
Dimensionless Index (LV/AV): 0.81

MR max vel: 451.0 cm/sec CO(LVOT): 3.4 l/min
MR max PG: 81.4 mmHg CI(LVOT): 1.9 l/min/m2
MR mean vel: 355.0 cm/sec SV(LVOT): 46.3 ml
MR mean PG: 55.0 mmHg SI(LVOT): 26.7 ml/m2
MR VTI: 156.0 cm
MR PISA radius: 0.50 cm
MR alias vel: 38.5 cm/sec
MR ERO: 0.13 cm2
MR volume: 20.9 ml
MR flow rate: 60.5 cm3/sec

PA V2 max: 73.0 cm/sec RV V1 max: 56.1 cm/sec

Procedure Note

PA max PG: 2.0 mmHg RV V1 max PG: 1.3 mmHg
RV S'(nl>=9.5cm/sec): 12.4 cm/sec

MR regurgitant fraction: 31.1 %

Reading Physician:Electronically signed by: Arvin Narula, MD on 01/14/2024 07:37 AM
Ordering Physician: JONES, CHRISTOPHER
Performed By: Rodil, Alla, RDCS

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	CV ECHO PROCEDURES	Edited Result - Final

BLOOD GAS ALL ANALYTES, ARTERIAL - Final result (01/13/2024 7:12 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.40	7.35 - 7.45		01/13/2024 7:16 AM PST	MERCY LAB CV	
pCO2, Arterial	42	35 - 45 mm Hg		01/13/2024 7:16 AM PST	MERCY LAB CV	
pO2, Arterial	286	83 - 108 mm Hg		01/13/2024 7:16 AM PST	MERCY LAB CV	
HCO3, Arterial	25.8	21.0 - 28.0 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
O2 Sat, Arterial	100.0	95.0 - 98.0 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	1.0	-2.0 - 3.0 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
FIO2, Arterial	100.0			01/13/2024 7:16 AM PST	MERCY LAB CV	
Oxygen Device	Ventilator			01/13/2024 7:16 AM PST	MERCY LAB CV	
Modes	PRVC			01/13/2024 7:16 AM PST	MERCY LAB CV	
Total Hgb, Whole Blood	11.1	11.9 - 15.3 g/dL		01/13/2024 7:16 AM PST	MERCY LAB CV	
Oxyhemoglobin, Arterial	98.4	95.0 - 98.0 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Carboxyhemoglobin, Arterial	0.4	1.0 - 1.5 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Methemoglobin, Arterial	1.2	<3.0 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Lactic Acid, Arterial	1.1	0.4 - 0.9 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose, Whole Blood	186	70 - 125 mg/dL		01/13/2024 7:16 AM PST	MERCY LAB CV	
Ionized Calcium, Whole Blood	1.17	1.15 - 1.27 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Sodium, Whole Blood	134	136 - 146 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Potassium, Whole Blood	5.3	3.5 - 5.1 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Chloride, Whole Blood	98	98 - 107 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Allen test	Positive			01/13/2024 7:16 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/13/2024 7:16 AM PST	MERCY LAB CV	
Comment	400/20/+8			01/13/2024 7:16 AM PST	MERCY LAB CV	
Operator ID	243,099			01/13/2024 7:16 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/13/2024 7:12 AM PST	01/13/2024 7:16 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/13/2024 6:59 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 1:42 PM PST	

Impressions

01/13/2024 1:43 PM PST
IMPRESSION: Endotracheal and gastric tubes as described below.

Unchanged large right pleural effusion

release

Narrative

01/13/2024 1:43 PM PST

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 6:59 AM

HISTORY: intubation

COMPARISON: 1/13/2024

FINDINGS:

Endotracheal tube terminates 4.6 cm above the carina. Gastric tube terminates in the proximal stomach.

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis or infiltrate. No pneumothorax.

Imaged skeletal structures are unchanged.

Procedure Note

Raymond Hong, MD - 01/13/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 6:59 AM

HISTORY: intubation

COMPARISON: 1/13/2024

FINDINGS:

Endotracheal tube terminates 4.6 cm above the carina. Gastric tube terminates in the proximal stomach.

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis or infiltrate. No pneumothorax.

Imaged skeletal structures are unchanged.

IMPRESSION:
IMPRESSION: Endotracheal and gastric tubes as described below.

Unchanged large right pleural effusion

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

GLUCOSE, POC - Final result (01/13/2024 6:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	142	70 - 125 mg/dL		01/13/2024 6:42 AM PST	MERCY LAB CV	
Operator ID	157,150			01/13/2024 6:42 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 6:40 AM PST	01/13/2024 6:42 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LIPID PANEL W/ REFLEX TO DIRECT LDL IF INDICATED - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/13/2024 1:02 PM PST	SCRIPPS LAB	
Triglycerides	72	7 - 150 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Comment:

```
Normal          <150      mg/dL
Borderline High  150-199  mg/dL
High             200-499  mg/dL
Very high       >=500     mg/dL

N-Acetyl-L-Cysteine at therapeutically achieved concentrations may lead to falsely low results.
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cholesterol	110	7 - 200 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Comment:

```
Desirable        <200       mg/dL
Borderline high   200-239   mg/dL
High             >= 240     mg/dL
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HDL Cholesterol	33	>=40 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Comment:

```
Major Risk Factor for Heart Disease
<40 mg/dL

Negative Risk Factor for Heart Disease
>= 60 mg/dL

N-acetyl-L-cysteine at elevated concentrations may lead to falsely low results
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
LDL Calculation	63	<100 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

SCRIPPS LAB - 01/13/2024 1:02 PM PST

Narrative

Reference ranges are derived from the clinical practice ATPIII Guidelines of the National Cholesterol Education Program (NCEP) on the Management of Blood Cholesterol: A Report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines, More recent guidelines using clinical and laboratory risk stratification approach provide more specific recommendations for specific patient groups (November 2018).

The relationship between levels of LDL cholesterol and Coronary Heart Disease is continuous over a broad range of LDL levels. A reduction in Cholesterol, Triglycerides and LDL levels has been associated with a reduced risk of CHD.

https://www.ahajournals.org/doi/10.1161/CIR.0000000000000625

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

HIV ANTIGEN/ANTIBODY COMBO - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HIV Antigen / Antibody Combo	Non Reactive	Non Reactive		01/13/2024 12:02 PM PST	SCRIPPS LAB	

Comment: HIV-1 p24 antigen and HIV-1/HIV-2 antibodies are not detected.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:24 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

HEMOGRAM - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	21.4	3.4 - 11.0 K/mcL		01/13/2024 5:34 AM PST	MERCY LAB CV	
RBC	3.83	3.98 - 5.25 M/mcL		01/13/2024 5:34 AM PST	MERCY LAB CV	
HGB	11.8	11.9 - 15.3 g/dL		01/13/2024 5:34 AM PST	MERCY LAB CV	
HCT	38.8	37.3 - 46.7 %		01/13/2024 5:34 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		01/13/2024 5:34 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/13/2024 5:34 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCHC	30.4	31.0 - 36.0 g/dL		01/13/2024 5:34 AM PST	MERCY LAB CV	
RDW	14.9	11.7 - 14.9 %		01/13/2024 5:34 AM PST	MERCY LAB CV	
Platelet Count	801	150 - 425 K/mcL		01/13/2024 5:34 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:24 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.014	<=0.034 ng/mL		01/13/2024 6:00 AM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population. This patient's result is considered normal. However, clinical
correlation is suggested to determine whether serial measurements are necessary for further evaluation.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CK - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
CK	148	30 - 135 Units/L		01/13/2024 5:55 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	235	70 - 125 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Sodium	133	137 - 145 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Potassium	6.0	3.5 - 5.1 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
CO2, Total	28	22 - 30 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Urea Nitrogen	55	7 - 17 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Creatinine	1.39	0.50 - 1.00 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	44	>60 mL/min/1.73m*2		01/13/2024 6:01 AM PST	MERCY LAB CV	
Anion Gap	4	6 - 14 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Osmolality Calculated	299	280 - 305 mOsm/kg H20		01/13/2024 6:01 AM PST	MERCY LAB CV	
Calcium	8.7	8.4 - 10.2 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Phosphorus	9.4	2.5 - 4.5 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Albumin	3.4	3.5 - 5.0 g/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BLOOD GAS, ARTERIAL - Final result (01/13/2024 5:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.10	7.35 - 7.45		01/13/2024 5:09 AM PST	MERCY LAB CV	
pCO2, Arterial	87	35 - 45 mm Hg		01/13/2024 5:09 AM PST	MERCY LAB CV	
pO2, Arterial	117	83 - 108 mm Hg		01/13/2024 5:09 AM PST	MERCY LAB CV	
HCO3, Arterial	27.2	21.0 - 28.0 mmol/L		01/13/2024 5:09 AM PST	MERCY LAB CV	
O2 Sat, Arterial	97.0	95.0 - 98.0 %		01/13/2024 5:09 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	-2.4	-2.0 - 3.0 mmol/L		01/13/2024 5:09 AM PST	MERCY LAB CV	
FIO2, Arterial	60.0			01/13/2024 5:09 AM PST	MERCY LAB CV	
Oxygen Device	BIPAP			01/13/2024 5:09 AM PST	MERCY LAB CV	
Modes	NA			01/13/2024 5:09 AM PST	MERCY LAB CV	
Allen test	NA			01/13/2024 5:09 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/13/2024 5:09 AM PST	MERCY LAB CV	
Comment	14/7/16			01/13/2024 5:09 AM PST	MERCY LAB CV	
Notified By	243560			01/13/2024 5:09 AM PST	MERCY LAB CV	
Notified Whom	157150			01/13/2024 5:09 AM PST	MERCY LAB CV	
Operator ID	243,560			01/13/2024 5:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/13/2024 5:01 AM PST	01/13/2024 5:09 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/13/2024 4:09 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 12:26 PM PST	

Impressions

01/13/2024 12:27 PM PST
IMPRESSION: No significant change.

release

Narrative

01/13/2024 12:27 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 4:09 AM

HISTORY: f/u thoracentesis

COMPARISON: 1/12/2024

FINDINGS:

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis. No pneumothorax.

Imaged skeletal structures are unchanged.

Procedure Note

Raymond Hong, MD - 01/13/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 4:09 AM

HISTORY: f/u thoracentesis

COMPARISON: 1/12/2024

FINDINGS:

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis. No pneumothorax.

Imaged skeletal structures are unchanged.

IMPRESSION:
IMPRESSION: No significant change.

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

BLOOD GAS LACTIC ACID, ARTERIAL - Final result (01/13/2024 2:55 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.13	7.35 - 7.45		01/13/2024 3:11 AM PST	MERCY LAB CV	
pCO2, Arterial	84	35 - 45 mm Hg		01/13/2024 3:11 AM PST	MERCY LAB CV	
pO2, Arterial	78	83 - 108 mm Hg		01/13/2024 3:11 AM PST	MERCY LAB CV	
HCO3, Arterial	28.2	21.0 - 28.0 mmol/L		01/13/2024 3:11 AM PST	MERCY LAB CV	
O2 Sat, Arterial	90.5	95.0 - 98.0 %		01/13/2024 3:11 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	-1.0	-2.0 - 3.0 mmol/L		01/13/2024 3:11 AM PST	MERCY LAB CV	
FIO2, Arterial	32.0			01/13/2024 3:11 AM PST	MERCY LAB CV	
Oxygen Device	NC			01/13/2024 3:11 AM PST	MERCY LAB CV	
Modes	NA			01/13/2024 3:11 AM PST	MERCY LAB CV	
Lactic Acid, Arterial	1.1	0.4 - 0.9 mmol/L		01/13/2024 3:11 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Allen test	NA			01/13/2024 3:11 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/13/2024 3:11 AM PST	MERCY LAB CV	
Comment	4L NC			01/13/2024 3:11 AM PST	MERCY LAB CV	
Notified By	243560			01/13/2024 3:11 AM PST	MERCY LAB CV	
Notified Whom	157150			01/13/2024 3:11 AM PST	MERCY LAB CV	
Operator ID	243,560			01/13/2024 3:11 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/13/2024 2:55 AM PST	01/13/2024 3:11 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PATHOLOGIST FLUID COMMENT - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Pathologist Review, CSF/ Body Fluids	Morphologic analysis of the smear shows predominantly acute inflammation. Yi Zhuang, M.D.			01/15/2024 9:24 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:07 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, WOUND (WOUND/FLUID/TISSUE CULTURE W/STAIN) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Wound Culture	No Growth Final		MIC	01/17/2024 8:53 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	Many WBCs			01/17/2024 8:53 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	No organisms seen			01/17/2024 8:53 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

BODY FLUID CELL COUNT WITH DIFF (BODY FLUID CELL COUNT WITH DIFFERENTIAL) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Volume (ML)	17.0	mL		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Color	Light Yellow			01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Appearance	Cloudy			01/13/2024 1:20 PM PST	MERCY LAB CV	
Comment: Cellular debris present may affect results.						
Fluid Nucleated Cell Count	103,752	/mcL		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid RBC Count	20,000	/mcL		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Neutrophils %	94	%		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Lymphocytes %	1	%		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Mononuclears %	5	%		01/13/2024 1:20 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Bacteria	Present	None Seen		01/13/2024 1:20 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:07 AM PST

Narrative

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MERCY LAB CV - 01/13/2024 1:20 PM PST
Reference Interval has not been established for this specimen type.
```

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTEIN BODY FLUID (PROTEIN, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/13/2024 3:48 AM PST	MERCY LAB CV	
Protein (FL)	4.5	Not Established g/dL		01/13/2024 3:48 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

```
MERCY LAB CV - 01/13/2024 3:48 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories.
The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for clinical purposes.
```

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTATE DEHYDROGENASE BODY FLUID (LACTATE DEHYDROGENASE, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/13/2024 4:06 AM PST	MERCY LAB CV	
Lactate Dehrdrogenase (FL)	1,379	Not Established Units/L		01/13/2024 4:06 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

```
MERCY LAB CV - 01/13/2024 4:06 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories.
The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for clinical purposes.
```

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE BODY FLUID (GLUCOSE, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/13/2024 3:48 AM PST	MERCY LAB CV	
Glucose (FL)	<20	Not Established mg/dL		01/13/2024 3:48 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

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MERCY LAB CV - 01/13/2024 3:48 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories.
The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for clinical purposes.
```

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

ALBUMIN BODY FLUID (ALBUMIN, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Albumin (FL)	2.0	Not Established g/dL		01/13/2024 3:48 AM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/13/2024 3:48 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

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MERCY LAB CV - 01/13/2024 3:48 AM PST
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Narrative
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

NON-GYN CYTOPATHOLOGY - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Case Report	Cytopathology Report - Non-gyn Case: CC24-00016 Authorizing Provider: Jones, Christopher W, MD Collected: 01/13/2024 0202 Ordering Location: Scripps Mercy Hospital Received: 01/15/2024 1031 Chula Vista ICU Pathologist: Zhang, Sha, MD Specimen: Pleural cavity, right, body fluid			01/16/2024 2:12 PM PST	MERCY LAB CV	
Diagnosis	A: Pleural cavity, right, body fluid, cytology: Satisfactory for evaluation Negative for malignancy Acute inflammation and histiocytes			01/16/2024 2:12 PM PST	MERCY LAB CV	Electronically signed by Zhang, Sha, MD, PhD on 1/16/2024 at 1412 PST
Microscopic Examination	The microscopic findings support the above interpretation.			01/16/2024 2:12 PM PST	TORREY PINES LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Gross Examination	A Specimen received: volume: 2.5 cc, color: cloudy beige with debris. Specimen is forwarded to Green cytology for thinprep, cytospin, and cell block preparation. Mm TIF: 13:13 Slides prepared: 1 ThinPrep Pap stained monolayer, 1 Diff-Quik stained cytospin slide and 1 H&E stained cell block.			01/16/2024 2:12 PM PST	TORREY PINES LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Disclaimer	Processing and primary screening were performed at Scripps Medical Laboratories, Torrey Pines, 10666 N. Torrey Pines Road, La Jolla, CA 92037. Histology technical services were performed at Scripps Anatomic Pathology Laboratory, 9834 Genesee Avenue, La Jolla, CA 92037 Microscopic examination was performed at San Diego Pathologists Medical Group, 435 H Street, Chula Vista, CA 91910. If immunohistochemical staining test(s) were performed on this case at Scripps Anatomic Pathology Laboratory, they were developed and their performance characteristics determined by Scripps Anatomic Pathology Laboratory (9834 Genesee Avenue, La Jolla, CA 92037). It has not been cleared or approved by the U.S. Food and Drug Administration. The FDA has determined that such approval is not necessary. This test is used for clinical purposes. It should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA) as qualified to perform high complexity clinical laboratory testing. All controls show appropriate reactivity for each antibody reported.			01/16/2024 2:12 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Clinical History	Dx: Oral thrush Pleural effusion Prolonged QT interval COPD exacerbation (CMS/HCC) Acute kidney injury (CMS/HCC) Acute respiratory failure with hypercapnia (CMS/HCC) 59 yo F h/o HFrEF, Asthma, presents with shortness of breath, found to have concern for empyema			01/16/2024 2:12 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/15/2024 10:31 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB PATHOLOGY/ CYTOLOGY ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284
TORREY PINES LAB	10666 North Torrey Pines Rd	La Jolla, CA 92037	858-554-9560

SURVEILLANCE MRSA CULTURE - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Staph Screen culture	NO Methicillin/ Oxacillin Resistant Staphylococcus aureus		MIC	01/14/2024 6:28 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasal structure / Unknown		01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

FUNGUS CULTURE - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Result	No Fungus Isolated Final		MIC	02/02/2024 7:51 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

AFB CULTURE W/STAIN - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
AFB Culture	No AFB isolated Final		MIC	03/09/2024 6:07 PM PST	SCRIPPS LAB MICROBIOLOGY	
AFB Smear	No AFB seen by fluorescent stain			03/09/2024 6:07 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

CHOLESTEROL,BODY FLUID (CHOLESTEROL, BODY FLUID) - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cholesterol Fluid Source	Pleural fluid			01/15/2024 11:53 AM PST	ARUP LAB	
Cholesterol (FL)	38	mg/dL		01/15/2024 11:53 AM PST	ARUP LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

```
INTERPRETIVE INFORMATION: Cholesterol, Body Fluid

For information on body fluid reference ranges and/or interpretive
guidance visit http://aruplab.com/bodyfluids/

This test was developed and its performance characteristics
determined by ARUP Laboratories. It has not been cleared or
approved by the US Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for
clinical purposes.
Performed By: ARUP Laboratories
500 Chipeta Way
Salt Lake City, UT 84108
Laboratory Director: Jonathan R. Genzen, MD, PhD
CLIA Number: 46D0523979
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Pleural Fluid			01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ARUP LAB	500 Chipeta Way	Salt Lake City, UT 84108-1221	800-522-2787

TRIGLYCERIDES BODY FLUID (TRIGLYCERIDES, BODY FLUIDS) - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Triglycerides Fluid Source	Pleural fluid			01/15/2024 11:53 AM PST	ARUP LAB	
Triglycerides (FL)	38	mg/dL		01/15/2024 11:53 AM PST	ARUP LAB	

Comment:

```
INTERPRETIVE INFORMATION: Triglycerides, Fluid

For information on body fluid reference ranges and/or interpretive
guidance visit http://aruplab.com/bodyfluids/

This test was developed and its performance characteristics
determined by ARUP Laboratories. It has not been cleared or
approved by the US Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for
clinical purposes.
Performed By: ARUP Laboratories
500 Chipeta Way
Salt Lake City, UT 84108
Laboratory Director: Jonathan R. Genzen, MD, PhD
CLIA Number: 46D0523979
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Pleural Fluid			01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ARUP LAB	500 Chipeta Way	Salt Lake City, UT 84108-1221	800-522-2787

12-LEAD ECG - Final result (01/13/2024 12:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	118	bpm			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	92	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	294	ms			SCRIPPS RADIOLOGY	
QTC	412	ms			SCRIPPS RADIOLOGY	
R AXIS	60	degrees			SCRIPPS RADIOLOGY	
T AXIS	225	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 12:14 AM PST	01/15/2024 8:57 AM PST

Impressions

SCRIPPS RADIOLOGY - 01/15/2024 8:57 AM PST
Atrial fibrillation with rapid ventricular response
Left ventricular hypertrophy with repolarization abnormality (Cornell product)

Narrative

This result has an attachment that is not available.

Procedure Note

Arvin Narula, MD - 01/15/2024
Formatting of this note might be different from the original.
IMPRESSION:
Atrial fibrillation with rapid ventricular response
Left ventricular hypertrophy with repolarization abnormality (Cornell product)

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

GLUCOSE, POC - Final result (01/13/2024 12:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	121	70 - 125 mg/dL		01/13/2024 12:05 AM PST	MERCY LAB CV	
Operator ID	157,150			01/13/2024 12:05 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 12:01 AM PST	01/13/2024 12:05 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/12/2024 9:48 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/17/2024 11:01 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of right hand / Unknown	Venipuncture / Unknown	01/12/2024 9:48 PM PST	01/12/2024 10:02 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/12/2024 9:30 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/17/2024 11:01 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Non-Lab Collection / Unknown	01/12/2024 9:30 PM PST	01/12/2024 10:03 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS MICROSCOPIC W/ REFLEX TO CULTURE - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC (UA)	10-20	0 - 2 / HPF		01/12/2024 9:43 PM PST	MERCY LAB CV	
RBC (UA)	6-10	0 - 2 / HPF		01/12/2024 9:43 PM PST	MERCY LAB CV	
Epithelial Cells, Squamous (UA)	Few	None / LPF		01/12/2024 9:43 PM PST	MERCY LAB CV	
Bacteria (UA)	Few	None / HPF		01/12/2024 9:43 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

FENTANYL, URINE - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fentanyl, Interpretation (Ur)	Negative	Negative		01/12/2024 9:43 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS, MACROSCOPIC, W/REFLEX TO MICROSCOPIC, W/REFLEX TO CULTURE (URINALYSIS, CULTURE IF INDICATED) - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Color (UA)	Yellow	Yellow		01/12/2024 9:37 PM PST	MERCY LAB CV	
Clarity (UA)	Slightly Cloudy	Clear		01/12/2024 9:37 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Specific Gravity (UA)	1.020	1.005 - 1.030		01/12/2024 9:37 PM PST	MERCY LAB CV	
pH (UA)	5.5	5.0 - 8.5 pH		01/12/2024 9:37 PM PST	MERCY LAB CV	
Protein (UA)	30	Negative mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	
Glucose (UA)	Negative	Negative mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	
Ketones (UA)	Negative	Negative mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	
Bilirubin (UA)	Negative	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Occult Blood (UA)	Moderate	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Leukocyte Esterase (UA)	Small	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Nitrite (UA)	Positive	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Urobilinogen (UA)	0.2	<2.0 mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RAPID DRUG SCREEN, URINE (DRUG SCREEN, URINE) - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Benzodiazepines (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Cocaine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Tetrahydrocannabinol (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 50						

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Opiates (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						
Methamphetamine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Barbiturates (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Amphetamine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Methadone (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Phencyclidine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 25						
Tricyclic Antidepressants (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 300						
Oxycodone (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

MERCY LAB CV - 01/12/2024 9:48 PM PST
This method provides screening results for medical purposes only. A more specific alternate chemical method must be used for a confirmed analytical result. Clinical considerations and professional judgement should be applied to any drug of abuse result, particularly when screening positive results are used.

Contact laboratory to order confirmatory testing.

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINE CULTURE - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Urine Culture	Multiple organisms present, probable skin/ urogenital contamination.		MIC	01/15/2024 8:35 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:35 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/12/2024 8:49 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/12/2024 8:52 PM PST	

Impressions

01/12/2024 8:53 PM PST
IMPRESSION:
Large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly.

release

Narrative

01/12/2024 8:53 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/12/2024 8:49 PM

HISTORY: sepsis

COMPARISON: 12/22/2021

FINDINGS:
One view obtained.

Large right pleural effusion with subjacent atelectasis of the right lung. Left lung appears clear but appears hyperinflated. No pneumothorax.

The cardiomediastinal silhouette is enlarged. The osseous structures demonstrate no acute abnormality.

Procedure Note

Anshuman Bansal, MD - 01/12/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/12/2024 8:49 PM

HISTORY: sepsis

COMPARISON: 12/22/2021

FINDINGS:
One view obtained.

Large right pleural effusion with subjacent atelectasis of the right lung. Left lung appears clear but appears hyperinflated. No pneumothorax.

The cardiomediastinal silhouette is enlarged. The osseous structures demonstrate no acute abnormality.

IMPRESSION:
IMPRESSION:
Large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly.

release

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

BLOOD GAS LACTIC ACID, VENOUS - Final result (01/12/2024 7:44 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Venous	7.26	7.35 - 7.45		01/12/2024 7:53 PM PST	MERCY LAB CV	
pCO2, Venous	66	38 - 50 mm Hg		01/12/2024 7:53 PM PST	MERCY LAB CV	
pO2, Venous	28	Not Established mm Hg		01/12/2024 7:53 PM PST	MERCY LAB CV	
HCO3, Venous	29.5	22.0 - 29.0 mmol/L		01/12/2024 7:53 PM PST	MERCY LAB CV	
O2 Sat, Venous	41.8	60.0 - 85.0 %		01/12/2024 7:53 PM PST	MERCY LAB CV	
Base Excess/Deficit, Venous	2.3	Not Established mmol/L		01/12/2024 7:53 PM PST	MERCY LAB CV	
Lactic Acid, Venous	2.9	0.9 - 2.5 mmol/L		01/12/2024 7:53 PM PST	MERCY LAB CV	
Notification Note	CRITICAL VPH/VPCO2			01/12/2024 7:53 PM PST	MERCY LAB CV	
Notified By	128296			01/12/2024 7:53 PM PST	MERCY LAB CV	
Notified Whom	EMMA 241391 ER RN			01/12/2024 7:53 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Operator ID	128,296			01/12/2024 7:53 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:44 PM PST	01/12/2024 7:46 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COVID-19, RAPID COMBO (COVID-19, RAPID (PCR/NAAT, COMBO)) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected		01/12/2024 8:53 PM PST	MERCY LAB CV	

Comment:

A Detected result is considered a positive result for that virus.
A positive SARS-CoV-2 result indicates that RNA from SARS-CoV-2 was detected and that the patient is considered infected and presumed contagious. A positive Influenza A or B result indicates that RNA from Influenza A or B, respectively, was detected and that the patient is considered infected and presumed contagious.

A Not Detected (negative) test result means that RNA was not present in the specimen above the limit of detection. However, it does not rule out the possibility of infection and should not be used as the sole basis for patient management decisions. Test parameters for asymptomatic patients have not been evaluated.

These results have been reported to the San Diego Dept. of Health Services as required by Title 17, California Code of regulations, Section 2500.
Test information for patients can be found at the following link: https://www.fda.gov/media/142192/download
Test information for providers can be found at the following link: https://www.fda.gov/media/142191/download

This SARS-CoV-2 Real Time RT-PCR test was performed under an Emergency Use Authorization granted by the FDA for the Roche Liat.

This test meets Covid-19 testing requirements for school and business testing criteria and is not an antibody or antigen test.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

INFLUENZA A/B COMBO - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Influenza A PCR	Not Detected	Not Detected		01/12/2024 8:53 PM PST	MERCY LAB CV	
Influenza B PCR	Not Detected	Not Detected		01/12/2024 8:53 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

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MERCY LAB CV - 01/12/2024 8:53 PM PST
These results have been reported to the San Diego Department of Health Services as required by Title 17,
California Code of Regulations, Section 2505.
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Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

THYROID STIMULATING HORMONE (ED) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Thyroid Stimulating Hormone	1.260	0.465 - 4.680 mcIU/mL		01/12/2024 8:31 PM PST	MERCY LAB CV	

Comment: High dose biotin supplementation (>1 mg/day) may falsely decrease results.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	13,700	Refer to the reference range comment. pg/mL		01/12/2024 8:12 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

NT-proBNP reference range: <=125 pg/mL

NT-proBNP testing aids in the diagnosis or exclusion of heart failure (HF) when used in conjunction with clinical history, symptoms, exam findings, and additional imaging and laboratory studies.

Emergency Department:
The ICON-RELOADED study provided validation of cutoff values for NT-proBNP in patients presenting with dyspnea in the Emergency Department (1).
In this setting NT-proBNP level <300 pg/mL has a high negative predictive value for HF. For values above 300 pg/mL, age-specific cutoffs provide additional decision limit categories:

Result.....Age.....Interpretation
<300.....All.....Negative: Heart Failure Unlikely
=300 to <450.....22-<50..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <900.....50-<75..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <1800....>=75....Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=>450.....22-<50..Positive: Heart Failure Likely
=>900.....50-<75..Positive: Heart Failure Likely
=>1800.....>=75....Positive: Heart Failure Likely

* Natriuretic peptides values in the gray zones may be caused by conditions other than heart failure. Acute coronary syndrome,pulmonary embolism,pulmonary hypertension,sepsis,stroke,and renal dysfunction will elevate NT- proBNP levels. Obesity, flash pulmonary edema,cardiac tamponade,and pericardial constriction are conditions associated with reduced NT-proBNP(2).

Ambulatory Clinics:
For patients presenting to outpatient clinics with suspicion of HF, the optimal clinical utility of NT-proBNP is to
exclude heart failure.For this clinical setting Ortho Clinical Diagnostics recommends NT-proBNP results be interpreted
as in the table below(2):

Result.....Age.....Interpretation
<125.....All.....Negative: Heart Failure Unlikely
>=125.....All.....Consider Heart Failure as well as other causes* of NT-proBNP elevation.

* Natriuretic peptides value elevations could also be caused by conditions other than heart failure such as acute
coronary syndrome, pulmonary embolism, pulmonary hypertension, sepsis, stroke, and renal dysfunction (2).

References:
1. Terminal Pro-B-Type Natriuretic Peptide in the Emergency Department: The ICON-RELOADED Study. J Am Coll Cardiol. 2018 Mar 20;71(11):1191-1200. doi: 10.1016/j.jacc.2018.01.021. PMID: 29544601.

2. NBNP2 Instructions For Use. Ortho Clinical Diagnostics

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROCALCITONIN - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Procalcitonin	0.29	0.03 - 0.08 ng/mL		01/13/2024 2:38 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

MERCY LAB CV - 01/13/2024 2:38 AM PST
Procalcitonin (PCT) is a dynamic biomarker with value in guiding antibiotic management in select clinical settings. PCT values are most useful when trends are analyzed. Decisions on antibiotic use should not be based solely on PCT level. Consider repeating PCT no more than daily in ICU/sepsis patients, and every 2 days for LRTI in order to discontinue antibiotics.

Suspected Lower Respiratory Tract Infection (LRTI):
0.1-0.25 ng/mL - low likelihood for bacterial infection, antibiotics discouraged
>0.25 ng/mL - increased likelihood for bacterial infection.

Suspected Sepsis
0.1-0.5 ng/mL - low likelihood for sepsis, antibiotics discouraged.
>0.5 ng/mL - increased likelihood of sepsis.
>2.0 ng/mL - high risk of sepsis/septic shock.

Consider discontinuation of antibiotics when PCT <0.25 (LRTI) or <0.5 (sepsis), or 80% reduction from baseline.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	18.7	3.4 - 11.0 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
RBC	4.40	3.98 - 5.25 M/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
HGB	13.2	11.9 - 15.3 g/dL		01/12/2024 9:40 PM PST	MERCY LAB CV	
HCT	43.7	37.3 - 46.7 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
MCV	99	81 - 100 fL		01/12/2024 9:40 PM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		01/12/2024 9:40 PM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		01/12/2024 9:40 PM PST	MERCY LAB CV	
RDW	14.8	11.7 - 14.9 %		01/12/2024 9:40 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	799	150 - 425 K/ mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
MPV	9.2	9.0 - 12.8 fL		01/12/2024 9:40 PM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/12/2024 9:40 PM PST	MERCY LAB CV	
Preliminary Abs Neut	16.76	1.50 - 7.40 K/ mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.029	<=0.034 ng/mL		01/12/2024 8:12 PM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population. This patient's result is considered normal. However, clinical
correlation is suggested to determine whether serial measurements are necessary for further evaluation.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MANUAL DIFFERENTIAL,REFLEXED FROM AUTO (MANUAL DIFFERENTIAL,REFLEXED) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Neutrophils %	93.0	38.0 - 74.0 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Bands %	3	0 - 6 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Lymphocytes %	2.0	16.0 - 48.0 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Monocytes %	2.0	4.9 - 12.5 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Eosinophils %	0.0	0.0 - 9.5 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Basophils %	0.0	0.0 - 1.6 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Neutrophils	17.39	1.50 - 7.40 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Bands	0.56	<=0.60 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Lymphocytes	0.37	0.90 - 3.10 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Monocytes	0.37	0.26 - 0.87 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Eosinophils	0.00	0.00 - 0.51 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Basophils	0.00	0.00 - 0.09 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
PLT Slide Review	Verified by slide scan.			01/12/2024 9:40 PM PST	MERCY LAB CV	
RBC Slide Review	Reviewed			01/12/2024 9:40 PM PST	MERCY LAB CV	
Ovalocytes	Few			01/12/2024 9:40 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

APTT (ACTIVATED PTT) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
aPTT	31	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds		01/12/2024 8:04 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTIME-INR (PROTHROMBIN TIME-INR) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protime	13.3	10.0 - 13.1 Seconds		01/12/2024 8:04 PM PST	MERCY LAB CV	
INR	1.1	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation		01/12/2024 8:04 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LIPASE - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Lipase	114	23 - 300 Units/L		01/13/2024 2:15 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/12/2024 8:01 PM PST	MERCY LAB CV	
Glucose	132	70 - 125 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Sodium	137	137 - 145 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Potassium	5.1	3.5 - 5.1 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Urea Nitrogen	48	7 - 17 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Creatinine	1.09	0.50 - 1.00 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	59	>60 mL/min/1.73m*2		01/12/2024 8:01 PM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Osmolality Calculated	298	280 - 305 mOsm/kg H20		01/12/2024 8:01 PM PST	MERCY LAB CV	
Calcium	9.6	8.4 - 10.2 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Protein, total	8.8	6.3 - 8.2 g/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Albumin	3.7	3.5 - 5.0 g/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Alkaline Phosphatase	133	38 - 126 Units/L		01/12/2024 8:01 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
AST	70	14 - 36 Units/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
ALT	31	<=34 Units/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Bilirubin, Total	0.8	0.2 - 1.3 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

MERCY LAB CV - 01/12/2024 8:01 PM PST

```
for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (01/12/2024 7:36 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	152	bpm			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	98	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	318	ms			SCRIPPS RADIOLOGY	
QTC	505	ms			SCRIPPS RADIOLOGY	
R AXIS	69	degrees			SCRIPPS RADIOLOGY	
T AXIS	212	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/12/2024 7:36 PM PST	01/15/2024 1:15 PM PST

Impressions

SCRIPPS RADIOLOGY - 01/15/2024 1:15 PM PST

Impressions

```
** Critical Test Result: High HR
Atrial fibrillation with rapid ventricular response
Minimal voltage criteria for LVH, may be normal variant ( Cornell product )
Septal infarct , age undetermined
Abnormal ECG
When compared with ECG of 22-Dec-2021 20:06,
Atrial fibrillation has replaced Sinus rhythm
Septal infarct is now present
Nonspecific T wave abnormality, worse in Inferior leads
```

Narrative

This result has an attachment that is not available.

Procedure Note

Stephen Zanoni, MD - 01/15/2024

Formatting of this note might be different from the original.

IMPRESSION:
** Critical Test Result: High HR
Atrial fibrillation with rapid ventricular response
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Septal infarct , age undetermined
Abnormal ECG
When compared with ECG of 22-Dec-2021 20:06,
Atrial fibrillation has replaced Sinus rhythm
Septal infarct is now present
Nonspecific T wave abnormality, worse in Inferior leads

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

BLOOD GAS, ARTERIAL - Final result (12/23/2021 1:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.44	7.35 - 7.45		12/23/2021 1:47 PM PST	MERCY LAB CV	
pCO2, Arterial	59	35 - 45 mm Hg		12/23/2021 1:47 PM PST	MERCY LAB CV	
pO2, Arterial	81	83 - 108 mm Hg		12/23/2021 1:47 PM PST	MERCY LAB CV	
HCO3, Arterial	40.0	21.0 - 28.0 mmol/L		12/23/2021 1:47 PM PST	MERCY LAB CV	
O2 Sat, Arterial	96.7	95.0 - 98.0 %		12/23/2021 1:47 PM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	15.9	-2.0 - 3.0 mmol/L		12/23/2021 1:47 PM PST	MERCY LAB CV	
Oxygen Device	Room Air			12/23/2021 1:47 PM PST	MERCY LAB CV	
Modes	NA			12/23/2021 1:47 PM PST	MERCY LAB CV	
Notified By	243446			12/23/2021 1:47 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Notified Whom	166427			12/23/2021 1:47 PM PST	MERCY LAB CV	
Operator ID	243,446			12/23/2021 1:47 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	12/23/2021 1:41 PM PST	12/23/2021 1:47 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
John-Vincent Arcilla MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (12/23/2021 5:20 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	8.0	3.4 - 11.0 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
RBC	3.82	3.98 - 5.25 M/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
HGB	11.9	11.9 - 15.3 g/dL		12/23/2021 8:07 AM PST	MERCY LAB CV	
HCT	38.0	37.3 - 46.7 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		12/23/2021 8:07 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		12/23/2021 8:07 AM PST	MERCY LAB CV	
MCHC	31.3	31.0 - 36.0 g/dL		12/23/2021 8:07 AM PST	MERCY LAB CV	
RDW	13.8	11.7 - 14.9 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Platelet Count	342	150 - 425 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
nRBC	0.0	0 /100 WBCs		12/23/2021 8:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	5.80	1.50 - 7.40 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Neutrophils %	72.4	38.0 - 74.0 %		12/23/2021 8:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
IG%	0.4	0.0 - 1.2 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Lymphocytes %	20.9	16.0 - 48.0 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Monocytes %	4.0	4.9 - 12.5 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Eosinophils %	1.8	0.0 - 9.5 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Basophils %	0.5	0.0 - 1.6 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	5.80	1.50 - 7.40 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.67	0.90 - 3.10 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.32	0.26 - 0.87 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.14	0.00 - 0.51 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/23/2021 5:20 AM PST	12/23/2021 7:54 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
John-Vincent Arcilla MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (12/23/2021 5:20 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/23/2021 6:13 AM PST	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	97	98 - 107 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
Glucose	109	70 - 125 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Urea Nitrogen	31	7 - 17 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Creatinine	0.9	0.5 - 1.0 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Osmolality Calculated	293	280 - 305 mOsm/ kg H20		12/23/2021 6:13 AM PST	MERCY LAB CV	
Albumin	3.1	3.5 - 5.0 g/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Protein, total	6.1	6.3 - 8.2 g/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Alkaline Phosphatase	83	38 - 126 Units/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
AST	38	14 - 36 Units/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
Bilirubin, Total	0.4	0.2 - 1.3 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
ALT	32	<=34 Units/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
GFR Calculation, African American	>60	>60 mL/ min/ 1.73m*2		12/23/2021 6:13 AM PST	MERCY LAB CV	
GFR Calculation, Non-African American	>60	>60 mL/ min/ 1.73m*2		12/23/2021 6:13 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/23/2021 5:20 AM PST	12/23/2021 5:42 AM PST

Narrative

MERCY LAB CV - 12/23/2021 6:13 AM PST
eGFR not calculated on patients <18 years of age.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RAPID DRUG SCREEN, URINE (DRUG SCREEN, URINE) - Final result (12/22/2021 10:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Benzodiazepines (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Cocaine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Tetrahydrocannabinol (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 50						
Opiates (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						
Methamphetamine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Barbiturates (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Amphetamine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Methadone (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Phencyclidine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 25						
Tricyclic Antidepressants (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 300						
Oxycodone (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen collection, clean catch / Unknown		12/22/2021 10:41 PM PST	12/22/2021 10:43 PM PST

Narrative

MERCY LAB CV - 12/22/2021 11:04 PM PST
This method provides screening results for medical purposes only. A more specific alternate chemical method must be used for a confirmed analytical result. Clinical considerations and professional judgement should be applied to any drug of abuse result, particularly when screening positive results are used.

Contact laboratory to order confirmatory testing.

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS DIPSTICK W/ REFLEX TO MICROSCOPIC (URINALYSIS WITH MICROSCOPIC IF INDICATED) - Final result (12/22/2021 10:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Color (UA)	Yellow	Yellow		12/22/2021 10:46 PM PST	MERCY LAB CV	
Clarity (UA)	Clear	Clear		12/22/2021 10:46 PM PST	MERCY LAB CV	
Specific Gravity (UA)	1.010	1.005 - 1.030		12/22/2021 10:46 PM PST	MERCY LAB CV	
pH (UA)	6.0	5.0 - 8.5 pH		12/22/2021 10:46 PM PST	MERCY LAB CV	
Protein (UA)	Negative	Negative mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	
Glucose (UA)	Negative	Negative mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	
Ketones (UA)	Negative	Negative mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	
Bilirubin (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Occult Blood (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Leukocyte Esterase (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Nitrite (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Urobilinogen (UA)	0.2	<2.0 mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen collection, clean catch / Unknown		12/22/2021 10:41 PM PST	12/22/2021 10:43 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COVID-19, RAPID COMBO (COVID-19, RAPID (PCR/NAAT, COMBO)) - Final result (12/22/2021 9:08 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected		12/22/2021 9:37 PM PST	MERCY LAB CV	

Comment:

A Detected result is considered a positive result for that virus.
A positive SARS-CoV-2 result indicates that RNA from SARS-CoV-2 was detected and that the patient is considered infected and presumed contagious. A positive Influenza A or B result indicates that RNA from Influenza A or B, respectively, was detected and that the patient is considered infected and presumed contagious.

A Not Detected (negative) test result means that RNA was not present in the specimen above the limit of detection. However, it does not rule out the possibility of infection and should not be used as the sole basis for patient management decisions. Test parameters for asymptomatic patients have not been evaluated.

These results have been reported to the San Diego Dept. of Health Services as required by Title 17, California Code of regulations, Section 2500.
Test information for patients can be found at the following link: https://www.fda.gov/media/142192/download
Test information for providers can be found at the following link: https://www.fda.gov/media/142191/download

This SARS-CoV-2 Real Time RT-PCR test was performed under an Emergency Use Authorization granted by the FDA for the Roche Liat.

This test meets Covid-19 testing requirements for school and business testing criteria and is not an antibody or antigen test.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		12/22/2021 9:08 PM PST	12/22/2021 9:11 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

INFLUENZA A/B COMBO - Final result (12/22/2021 9:08 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Influenza A PCR	Not Detected	Not Detected		12/22/2021 9:37 PM PST	MERCY LAB CV	
Influenza B PCR	Not Detected	Not Detected		12/22/2021 9:37 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		12/22/2021 9:08 PM PST	12/22/2021 9:11 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (12/22/2021 8:29 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/22/2021 8:37 PM PST	

Impressions

12/22/2021 8:39 PM PST
IMPRESSION:

Left midlung and retrocardiac opacities likely reflect atelectasis, although a developing infection cannot be completely excluded. Right lung is clear. No pulmonary edema.

Moderate cardiomegaly.

Small left pleural effusion. No pneumothorax.

Narrative

12/22/2021 8:39 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/22/2021 8:29 PM

HISTORY: chest pain

COMPARISON: None.

FINDINGS:

See impression.

Procedure Note

Christopher Maxwell, MD - 12/22/2021

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/22/2021 8:29 PM

HISTORY: chest pain

COMPARISON: None.

FINDINGS:

See impression.

IMPRESSION:
IMPRESSION:

Left midlung and retrocardiac opacities likely reflect atelectasis, although a developing infection cannot be completely excluded. Right lung is clear. No pulmonary edema.

Moderate cardiomegaly.

Small left pleural effusion. No pneumothorax.

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	9,890.0	<=125.0 pg/mL		12/22/2021 9:19 PM PST	MERCY LAB CV	

Comment:

```
Optimal Cut-Points (pg/mL)
<50 years old:  <= 450
50-75 years old: <= 900
>75 years old: <= 1800
Reference: Januzzi JL et. al.
European Heart Journal. 2006, 27: 330-337

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	9.2	3.4 - 11.0 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
RBC	3.91	3.98 - 5.25 M/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
HGB	12.1	11.9 - 15.3 g/dL		12/22/2021 8:54 PM PST	MERCY LAB CV	
HCT	39.3	37.3 - 46.7 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		12/22/2021 8:54 PM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		12/22/2021 8:54 PM PST	MERCY LAB CV	
MCHC	30.8	31.0 - 36.0 g/dL		12/22/2021 8:54 PM PST	MERCY LAB CV	
RDW	13.5	11.7 - 14.9 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Platelet Count	348	150 - 425 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
nRBC	0.0	0 /100 WBCs		12/22/2021 8:54 PM PST	MERCY LAB CV	
Preliminary Abs Neut	6.74	1.50 - 7.40 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Neutrophils %	73.7	38.0 - 74.0 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Lymphocytes %	19.3	16.0 - 48.0 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Monocytes %	4.2	4.9 - 12.5 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Eosinophils %	1.5	0.0 - 9.5 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Basophils %	0.5	0.0 - 1.6 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Neutrophils	6.74	1.50 - 7.40 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
IG#	0.07	0.00 - 0.10 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	1.77	0.90 - 3.10 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Monocytes	0.38	0.26 - 0.87 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Eosinophils	0.14	0.00 - 0.51 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.035	<=0.034 ng/mL		12/22/2021 9:19 PM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population.  This patient's result is suspicious for myocardial injury. However,
serial measurements may be necessary to confirm or exclude the diagnosis of acute myocardial infarction.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TSH (THYROID STIMULATING HORMONE) - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Thyroid Stimulating Hormone	0.679	0.465 - 4.680 mcIU/mL		12/23/2021 2:20 AM PST	MERCY LAB CV	

Comment: High dose biotin supplementation (>1 mg/day) may falsely decrease results.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LIPID PANEL (LIPID PANEL WITH CALC LDL) - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/23/2021 10:28 AM PST	SCRIPPS LAB	
Triglycerides	59	<150 mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	

Comment:

```
Normal          <150     mg/dL
Borderline High  150-199 mg/dL
High             200-499 mg/dL
Very high       >499     mg/dL
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cholesterol	122	<200 mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	

Comment:

```
Desirable        <200     mg/dL
Borderline High   200-239 mg/dL
High             >239     mg/dL
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HDL Cholesterol	62	Not Established mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	

Comment:

```
Low  <40 mg/dL
High >59 mg/dL(Negative Risk Factor)
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
LDL Calculation	48	<130 mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

SCRIPPS LAB - 12/23/2021 10:28 AM PST

Narrative

Reference ranges are derived from the clinical practice ATPIII Guidelines of the National Cholesterol Education Program (NCEP) for the prevention and management of high cholesterol in adults (September 2002).

The relationship between levels of LDL cholesterol and Coronary Heart Disease is continuous over a broad range of LDL levels. A reduction in Cholesterol, Triglycerides and LDL levels has been associated with a reduced risk of CHD.

http://www.nhlbi.nih.gov/guidelines/cholesterol/index.htm

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

BASIC METABOLIC PANEL - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/22/2021 9:07 PM PST	MERCY LAB CV	
Sodium	139	137 - 145 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Potassium	4.1	3.5 - 5.1 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
CO2, Total	35	22 - 30 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Urea Nitrogen	28	7 - 17 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
Creatinine	0.7	0.5 - 1.0 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
Glucose	109	70 - 125 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
Osmolality Calculated	294	280 - 305 mOsm/kg H20		12/22/2021 9:07 PM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
GFR Calculation, Non-African American	>60	>60 mL/min/1.73m*2		12/22/2021 9:07 PM PST	MERCY LAB CV	
GFR Calculation, African American	>60	>60 mL/min/1.73m*2		12/22/2021 9:07 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

MERCY LAB CV - 12/22/2021 9:07 PM PST
eGFR not calculated on patients <18 years of age.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (12/22/2021 8:06 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	113	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	113	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	146	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	84	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	340	ms			SCRIPPS RADIOLOGY	
QTC	466	ms			SCRIPPS RADIOLOGY	
P AXIS	62	degrees			SCRIPPS RADIOLOGY	
R AXIS	40	degrees			SCRIPPS RADIOLOGY	
T AXIS	90	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/22/2021 8:06 PM PST	12/23/2021 7:27 AM PST

Impressions

SCRIPPS RADIOLOGY - 12/23/2021 7:28 AM PST
Sinus tachycardia
Possible Left atrial enlargement
Left ventricular hypertrophy with repolarization abnormality (Cornell product)
Abnormal ECG
No previous ECGs available

Narrative

This result has an attachment that is not available.

Procedure Note

Douglas Ballesteros, MD - 12/23/2021
Formatting of this note might be different from the original.
IMPRESSION:
Sinus tachycardia
Possible Left atrial enlargement
Left ventricular hypertrophy with repolarization abnormality (Cornell product)
Abnormal ECG
No previous ECGs available

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers

Document Coverage Dates

Jul. 29, 1964 - Oct. 13, 2026

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

If you take your Lucy record on a thumb drive to a different doctor, he or she might be able to use his computer to read the file electronically. Your downloaded, machine-readable Personal Health Summary document is in a format called "CDA." If your doctor has a computer that understands CDA, your information is a folder on your thumb drive called **MachineReadable_XDMFormat**. You might need to enter a password before your doctor can use this file.

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Elizabeth Tina Petruccio

Continuity of Care Document, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Encounters - as of 10/13/2026

Date	Type	Department	Care Team	Description
06/22/2025 10:22 PM PDT - 06/23/2025 3:28 AM PDT	Emergency	Scripps Mercy Hospital Chula Vista Emergency Department 435 H Street Chula Vista, CA 91910-4307 619-691-7000	Miguel Mendoza Navarro, MD	Discharge Disposition: Disch/ Trans Other HC Inst
12/26/2024 3:45 PM PST	Telemedicine	San Diego Cardiac Care 4060 4th Ave Suite 650 SAN DIEGO, CA 92103-2118 858-900-9778	Jerrold Glassman, MD	
12/24/2024 12:34 AM PST - 12/24/2024 4:05 AM PST	Emergency	Scripps Mercy Hospital Chula Vista Emergency Department 435 H Street Chula Vista, CA 91910-4307 619-691-7000	Douglas Ballesteros, MD	Discharge Disposition: Disch to Home (Routine)
10/30/2024 2:30 PM PDT	Telemedicine	San Diego Cardiac Care 4060 4th Ave Suite 650 SAN DIEGO, CA 92103-2118 858-900-9778	Jerrold Glassman, MD	
09/23/2024 10:26 AM PDT - 09/23/2024 4:26 PM PDT	Emergency	Scripps Mercy Hospital Chula Vista Emergency Department 435 H Street Chula Vista, CA 91910-4307 619-691-7000	Mark Hernandez, MD	Discharge Disposition: Disch to Home (Routine)
09/18/2024 3:30 PM PDT	Office Visit	San Diego Cardiac Care 4060 4th Ave Suite 650 SAN DIEGO, CA 92103-2118 858-900-9778	Jerrold Glassman, MD	
08/14/2024 3:00 PM PDT	Consult	San Diego Cardiac Care 4060 4th Ave Suite 650 SAN DIEGO, CA 92103-2118 858-900-9778	Jerrold Glassman, MD	
03/15/2024	SNF Visit	Scripps Clinic Torrey Pines Gerontology 10710 N Torrey Pines Rd La Jolla, CA 92037-1027 858-554-6158	Sheila Bhavsar, MD	
03/13/2024	SNF Visit	Scripps Clinic Torrey Pines Gerontology 10710 N Torrey Pines Rd La Jolla, CA 92037-1027 858-554-6158	Sheila Bhavsar, MD	
03/12/2024	Documentation	Scripps Health Transition Care	Kimberly P	
03/11/2024	SNF Visit	Scripps Clinic Torrey Pines Gerontology 10710 N Torrey Pines Rd La Jolla, CA 92037-1027 858-554-6158	Sheila Bhavsar, MD	
03/08/2024	SNF Visit	Scripps Clinic Torrey Pines Gerontology 10710 N Torrey Pines Rd La Jolla, CA 92037-1027 858-554-6158	Sheila Bhavsar, MD	
03/07/2024	Documentation	Scripps Health Transition Care	Kimberly P	
03/06/2024	SNF Visit	Scripps Clinic Torrey Pines Gerontology 10710 N Torrey Pines Rd La Jolla, CA 92037-1027	Sheila Bhavsar, MD	

Date	Type	Department	Care Team	Description
		858-554-6158		
03/05/2024	Documentation	Scripps Health Transition Care	Kimberly P	
03/05/2024	SNF Visit	Scripps Clinic Torrey Pines Gerontology 10710 N Torrey Pines Rd La Jolla, CA 92037-1027 858-554-6158	Sheila Bhavsar, MD	
02/29/2024	Documentation	Scripps Health Transition Care	Kimberly P	Kimberly P
02/27/2024	SNF Visit	Scripps Clinic Torrey Pines Gerontology 10710 N Torrey Pines Rd La Jolla, CA 92037-1027 858-554-6158	Sheila Bhavsar, MD	
02/26/2024	Documentation	Scripps Health Transition Care	Kimberly P	Kimberly P
02/26/2024	SNF Visit	Scripps Clinic Torrey Pines Gerontology 10710 N Torrey Pines Rd La Jolla, CA 92037-1027 858-554-6158	Sheila Bhavsar, MD	
02/22/2024	SNF Visit	Scripps Clinic Torrey Pines Gerontology 10710 N Torrey Pines Rd La Jolla, CA 92037-1027 858-554-6158	Sheila Bhavsar, MD	
02/22/2024	Documentation	Scripps Health Transition Care	Kimberly P	Kimberly P
02/20/2024	Documentation	Scripps Health Transition Care	Kimberly P	Kimberly P
02/19/2024	SNF Visit	Scripps Clinic Torrey Pines Gerontology 10710 N Torrey Pines Rd La Jolla, CA 92037-1027 858-554-6158	Sheila Bhavsar, MD	
01/12/2024 7:16 PM PST - 02/17/2024 4:00 PM PST	Hospital Encounter	Scripps Mercy Hospital Chula Vista 4th FL Definitive Observation Unit 435 H Street Chula Vista, CA 91910-4307 619-691-7000	Stephen Zanoni, MD Christopher Jones, MD Pedro Dammert Coronado, MD Truc Ho, MD Martha Strauser, MD Diep Dao, DO Omar Khayyat, MD Amanda de Four, MD	Discharge Disposition: Disch to SNF
12/22/2021 7:41 PM PST - 12/23/2021 6:19 PM PST	Hospital Encounter	Scripps Mercy Hospital Chula Vista 4th FL Definitive Observation Unit 435 H Street Chula Vista, CA 91910-4307 619-691-7000	Douglas Ballesteros, MD Mark Lazarus, MD John-Vincent Arcilla, MD	Discharge Disposition: Disch to Home (Routine)

Allergies - as of 10/13/2026

Active Allergy	Reactions	Criticality	Noted Date	Comments
Lactose	Other (see comments)	Low	01/20/2024	Patient having diarrhea; only fluid milk - all other lactose items ok
Clindamycin	Other (see comments)		09/23/2024	Patient unsure of what reaction she has from medication.

Inactive Allergy	Reactions	Criticality	Noted Date	Inactivity Date	Comments
Lincosamides	Other (see comments), Unknown Reaction		07/11/2012	09/23/2024	

Medications - as of 10/13/2026

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.						Active
ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.				02/16/2024		Active
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			02/17/2024		Active
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3		08/14/2024		Active
gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule			09/18/2024		Active
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule			09/23/2024		Active
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024		Active
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	INHALE 2 PUFFS EVERY 4 (FOUR) HOURS AS NEEDED FOR WHEEZING. CALOPTIMA MCAL ID 93931022D30310	90 g	3		10/15/2024		Active
potassium chloride (MICRO-K) 10 mEq CR capsule	TAKE 1 CAPSULE BY MOUTH EVERY DAY	90 capsule	3		11/02/2025		Active
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	TAKE 1 TABLET BY MOUTH TWICE A DAY	180 tablet	3		11/02/2025		Active
furosemide (LASIX) 40 mg tablet	TAKE 1 TABLET BY MOUTH EVERY DAY	90 tablet	1		11/03/2025		Active
metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	TAKE 1 TABLET (25 MG TOTAL) BY MOUTH DAILY.	90 tablet			11/04/2025		Active

Active Problems - as of 10/13/2026

Problem	Noted Date	Diagnosed Date
Gallstones	02/18/2024	
Gall bladder pain	02/18/2024	
Pus collection in space between lung and chest wall	01/26/2024	
Acute osteomyelitis of cervical spine	01/26/2024	
Heart failure	01/13/2024	
Loculated pleural effusion	01/13/2024	
Atrial fibrillation (irregular heartbeat)	01/13/2024	
Respiratory insufficiency	01/12/2024	
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Resolved Problems - as of 10/13/2026

Problem	Noted Date	Diagnosed Date	Resolved Date
Severe sepsis	02/18/2024		03/24/2024

Immunizations - as of 10/13/2026

Immunization	Administration Dates	Next Due
Tdap greater than or equal to 7yrs IM	02/09/2024	

Social History - as of 10/13/2026

Smoking Status as of 06/22/2025

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Passive Smoke Exposure: Past				
Smokeless Tobacco: Never				

Smoking Status as of 09/18/2024

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Smokeless Tobacco: Never				

Smoking Status as of 01/30/2024

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Smokeless Tobacco: Never				

Patient History	Answer	Date Recorded
Alcohol Use Status	Not Currently	04/18/2024
Oz/Week (Alcohol Misuse Male >= 9 OR Female >= 4.8)	Not on file	04/18/2024

Housing Stability	Answer	Date Recorded
What is your living situation today?	Do not have a stable place to live	01/19/2024

Food Insecurity	Answer	Date Recorded
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024

Transportation Needs	Answer	Date Recorded
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

Travel History	Travel Start	Travel End
Mexico	05/22/2025	06/22/2025
Mexico	08/23/2024	09/23/2024
Mexico	12/12/2023	01/12/2024
Mexico	11/22/2021	12/22/2021

Last Filed Vital Signs - as of 10/13/2026

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	104/68	06/23/2025 3:04 AM PDT	
Pulse	89	06/23/2025 3:15 AM PDT	
Temperature	36.6 °C (97.9 °F)	06/23/2025 3:04 AM PDT	
Respiratory Rate	21	06/23/2025 3:15 AM PDT	
Oxygen Saturation	96%	06/23/2025 3:15 AM PDT	
Inhaled Oxygen Concentration	-	-	
Weight	65.9 kg (145 lb 4.5 oz)	06/22/2025 10:53 PM PDT	
Height	167.6 cm (5' 6")	06/22/2025 10:16 PM PDT	
Body Mass Index	23.45	06/22/2025 10:16 PM PDT	

Plan of Treatment - as of 10/13/2026

Not on file

Procedures - as of 10/13/2026

Procedure Name	Priority	Date/Time	Associated Diagnosis	Comments
Echocardiogram 2D With Definity Protocol As Needed (ECHOCARDIOGRAM 2D (WITH DEFINITY PROTOCOL/COLOR DOPPLER/SPECTRAL DOPPLER/ STRAIN/3D PRN))	Routine	09/18/2024 2:54 PM PDT		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/12/2024 9:50 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/12/2024 12:43 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/11/2024 11:27 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/11/2024 2:04 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/09/2024 11:17 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/09/2024 1:17 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/08/2024 10:52 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/07/2024 10:57 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/07/2024 10:03 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/06/2024 10:34 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/06/2024 10:59 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/05/2024 11:32 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/04/2024 11:08 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/04/2024 10:04 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/04/2024 1:55 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/03/2024 10:18 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/02/2024 11:30 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/01/2024 10:30 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/01/2024 2:11 AM PST		
RESPIRATORY THERAPY EVAL AND TREAT PROTOCOL	Routine	01/31/2024 10:38 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/31/2024 7:04 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/31/2024 10:47 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/30/2024 7:17 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/30/2024 7:16 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/30/2024 11:02 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/29/2024 9:31 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/28/2024 8:01 PM PST		

Procedure Name	Priority	Date/Time	Associated Diagnosis	Comments
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/28/2024 9:51 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/27/2024 11:32 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/27/2024 9:43 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/27/2024 3:30 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/26/2024 12:37 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/26/2024 12:25 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/25/2024 3:06 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/24/2024 10:57 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/24/2024 11:23 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/23/2024 10:29 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/23/2024 12:16 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/23/2024 10:38 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/23/2024 12:50 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/22/2024 9:27 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/21/2024 9:52 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/21/2024 9:14 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/20/2024 11:45 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/20/2024 12:54 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/19/2024 10:13 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/18/2024 11:00 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/18/2024 12:38 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/17/2024 12:53 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/17/2024 6:43 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/17/2024 6:42 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/17/2024 4:50 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/16/2024 12:29 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/16/2024 12:37 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/16/2024 12:35 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/16/2024 12:30 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/15/2024 9:43 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/14/2024 11:21 PM PST		

Procedure Name	Priority	Date/Time	Associated Diagnosis	Comments
EXTUBATION	Routine	01/14/2024 11:13 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/14/2024 7:09 AM PST		
VENTILATOR	Routine	01/13/2024 11:12 PM PST		
TUBE THORACOSTOMY INCLUDES WATER SEAL	Routine	01/13/2024 11:44 AM PST	Loculated pleural effusion	
Echocardiogram 2D With Definity Protocol As Needed (ECHOCARDIOGRAM 2D (WITH DEFINITY PROTOCOL/COLOR DOPPLER/SPECTRAL DOPPLER/ STRAIN/3D PRN))	Routine	01/13/2024 9:44 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/13/2024 7:47 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/13/2024 7:22 AM PST		
VENTILATOR	Routine	01/13/2024 7:00 AM PST		
VENTILATOR	Routine	01/13/2024 7:00 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/13/2024 4:41 AM PST		

Results - as of 10/13/2026

CT lower extremity left without contrast (CT LOWER EXTREMITY LEFT WITHOUT CONTRAST) - Final result (06/23/2025 12:30 AM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities	Left	Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			06/23/2025 12:38 AM PDT	

Impressions

06/23/2025 10:12 AM PDT
IMPRESSION:
Large subcutaneous hematoma in the left mid calf measuring up to 15.8 cm. No acute osseous abnormality.

release

Narrative

06/23/2025 10:12 AM PDT
EXAMINATION: CT LOWER EXTREMITY LEFT WITHOUT CONTRAST

DATE/TIME: 6/23/2025 12:30 AM

HISTORY: L lower extremity pain/swelling

COMPARISON: Radiographs from 6/22/2025

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:
EXTREMITY: Left lower leg
Mild diffuse subcutaneous edema.
There is a large blood attenuation fluid collection in the subcutaneous soft tissues of the left mid calf measuring approximately 4.3 x 8.1 x 15.8 cm
No acute fracture

EXPOSURE:
This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 2; Series: 7; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 9; DLP: 202
Exposure: 1; Series: 4; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 8; DLP: 529

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

Procedure Note

Jeffrey Rudie, MD, PhD - 06/23/2025

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: CT LOWER EXTREMITY LEFT WITHOUT CONTRAST

DATE/TIME: 6/23/2025 12:30 AM

HISTORY: L lower extremity pain/swelling

COMPARISON: Radiographs from 6/22/2025

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:
EXTREMITY: Left lower leg
Mild diffuse subcutaneous edema.
There is a large blood attenuation fluid collection in the subcutaneous soft tissues of the left mid calf measuring approximately 4.3 x 8.1 x 15.8 cm
No acute fracture

EXPOSURE:
This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 2; Series: 7; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 9; DLP: 202
Exposure: 1; Series: 4; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 8; DLP: 529

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

IMPRESSION:
IMPRESSION:
Large subcutaneous hematoma in the left mid calf measuring up to 15.8 cm. No acute osseous abnormality.

release

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	IMG CT PROCEDURES	Final Result

X-ray tibia and fibula left 1 or 2 views (X-RAY TIBIA AND FIBULA LEFT 1 OR 2 VIEWS) - Final result (06/22/2025 11:03 PM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities, Lower Leg	Left	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			06/23/2025 8:05 AM PDT	

Impressions

06/23/2025 8:06 AM PDT
IMPRESSION: No fracture or periosteal process appreciated.
2. Soft tissue lucency in the calf consistent with fluid collection

release

Narrative

06/23/2025 8:06 AM PDT

Narrative

EXAMINATION: X-RAY TIBIA AND FIBULA LEFT 1 OR 2 VIEWS

DATE/TIME:6/22/2025 11:03 PM

HISTORY: LLE pain

COMPARISON: None.

FINDINGS:

BONE: No apparent bony abnormality.
SOFT TISSUE: Soft tissue lucency in the posterior calf region
OTHER: Negative.

Procedure Note

Trevor Nelson, MD - 06/23/2025
Formatting of this note might be different from the original.
EXAMINATION: X-RAY TIBIA AND FIBULA LEFT 1 OR 2 VIEWS

DATE/TIME:6/22/2025 11:03 PM

HISTORY: LLE pain

COMPARISON: None.

FINDINGS:

BONE: No apparent bony abnormality.
SOFT TISSUE: Soft tissue lucency in the posterior calf region
OTHER: Negative.

IMPRESSION:
IMPRESSION: No fracture or periosteal process appreciated.
2. Soft tissue lucency in the calf consistent with fluid collection

release

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CULTURE, BLOOD (BLOOD CULTURE) - Final result (06/22/2025 10:52 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			06/28/2025 12:01 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of left upper limb / Unknown	Venipuncture / Unknown	06/22/2025 10:52 PM PDT	06/22/2025 11:03 PM PDT

Narrative

MERCY LAB CV - 06/28/2025 12:01 AM PDT
Reference range: No growth

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	7.7	3.4 - 11.0 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
RBC	2.70	3.98 - 5.25 M/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
HGB	8.5	11.9 - 15.3 g/dL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
HCT	26.2	37.3 - 46.7 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MCV	97	81 - 100 fL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MCH	32	26 - 33 pg		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MCHC	32.4	31.0 - 36.0 g/dL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
RDW	12.8	11.7 - 14.9 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Platelet Count	387	150 - 425 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MPV	8.5	9.0 - 12.8 fL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Neutrophils %	56.4	38.0 - 74.0 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
IG%	0.5	0.0 - 1.2 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Lymphocytes %	27.8	16.0 - 48.0 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Monocytes %	8.5	4.9 - 12.5 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Eosinophils %	6.1	0.0 - 9.5 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Neutrophils	4.33	1.50 - 7.40 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	2.13	0.90 - 3.10 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Monocytes	0.65	0.26 - 0.87 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Eosinophils	0.47	0.00 - 0.51 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Preliminary Abs Neut	4.33	1.50 - 7.40 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			06/28/2025 12:01 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous line / Unknown		06/22/2025 10:49 PM PDT	06/22/2025 11:03 PM PDT

Narrative

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MERCY LAB CV - 06/28/2025 12:01 AM PDT
Reference range: No growth
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Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

APTT (ACTIVATED PTT) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
aPTT	33	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds		06/22/2025 11:32 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTIME-INR (PROTHROMBIN TIME-INR) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protime	10.4	10.0 - 13.1 Seconds		06/22/2025 11:32 PM PDT	MERCY LAB CV	
INR	0.9	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation		06/22/2025 11:32 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTIC ACID, SERUM (LACTIC ACID, VENOUS) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Lactic Acid, Venous	1.20	0.70 - 2.10 mmol/L		06/22/2025 11:13 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:00 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				06/22/2025 11:24 PM PDT	MERCY LAB CV	
Glucose	87	70 - 125 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Sodium	142	137 - 145 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Potassium	4.7	3.5 - 5.1 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Chloride	106	98 - 107 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
CO2, Total	24	22 - 30 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Urea Nitrogen	48	7 - 17 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Creatinine	2.10	0.50 - 1.00 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	
CKD-EPI eGFR 2021	26	>60 mL/min/1.73m*2		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Anion Gap	12	6 - 14 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Osmolality Calculated	306	280 - 305 mOsm/kg H20		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

MERCY LAB CV - 06/22/2025 11:24 PM PDT

Narrative

```
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (12/24/2024 2:46 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.053	<=0.034 ng/mL		12/24/2024 3:20 AM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population.  This patient's result is suspicious for myocardial injury. However,
serial measurements may be necessary to confirm or exclude the diagnosis of acute myocardial infarction.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 2:46 AM PST	12/24/2024 2:50 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (12/24/2024 1:44 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/24/2024 8:10 AM PST	

Impressions

```
12/24/2024 8:50 AM PST
IMPRESSION: Similar exam with no acute cardiac or pulmonary disease appreciated


release
```

Narrative

12/24/2024 8:50 AM PST

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/24/2024 1:44 AM

HISTORY: chest pain

COMPARISON: 9/23/2024

FINDINGS:

LUNGS: Normal. No infiltrates or other abnormalities.
VASCULATURE: Normal. Unremarkable pulmonary vasculature.
CARDIAC: Normal. No cardiac silhouette abnormality.
MEDIASTINUM: Normal. No visible mass or adenopathy.
PLEURA: Normal. No effusion or pleural thickening.
BONES: Normal. No fracture or visible bony lesion.
OTHER: Negative.

Procedure Note

Trevor Nelson, MD - 12/24/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/24/2024 1:44 AM

HISTORY: chest pain

COMPARISON: 9/23/2024

FINDINGS:

LUNGS: Normal. No infiltrates or other abnormalities.
VASCULATURE: Normal. Unremarkable pulmonary vasculature.
CARDIAC: Normal. No cardiac silhouette abnormality.
MEDIASTINUM: Normal. No visible mass or adenopathy.
PLEURA: Normal. No effusion or pleural thickening.
BONES: Normal. No fracture or visible bony lesion.
OTHER: Negative.

IMPRESSION:
IMPRESSION: Similar exam with no acute cardiac or pulmonary disease appreciated

release

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	1,560	Refer to the reference range comment. pg/mL		12/24/2024 1:31 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

NT-proBNP reference range: <=125 pg/mL

NT-proBNP testing aids in the diagnosis or exclusion of heart failure (HF) when used in conjunction with clinical history, symptoms, exam findings, and additional imaging and laboratory studies.

Emergency Department:
The ICON-RELOADED study provided validation of cutoff values for NT-proBNP in patients presenting with dyspnea in the Emergency Department (1).
In this setting NT-proBNP level <300 pg/mL has a high negative predictive value for HF. For values above 300 pg/mL, age-specific cutoffs provide additional decision limit categories:

Result.....Age.....Interpretation
<300.....All.....Negative: Heart Failure Unlikely
=300 to <450.....22-<50..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <900.....50-<75..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <1800....>=75....Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=>450.....22-<50..Positive: Heart Failure Likely
=>900.....50-<75..Positive: Heart Failure Likely
=>1800.....>=75....Positive: Heart Failure Likely

* Natriuretic peptides values in the gray zones may be caused by conditions other than heart failure. Acute coronary syndrome,pulmonary embolism,pulmonary hypertension,sepsis,stroke,and renal dysfunction will elevate NT- proBNP levels. Obesity, flash pulmonary edema,cardiac tamponade,and pericardial constriction are conditions associated with reduced NT-proBNP(2).

Ambulatory Clinics:
For patients presenting to outpatient clinics with suspicion of HF, the optimal clinical utility of NT-proBNP is to
exclude heart failure.For this clinical setting Ortho Clinical Diagnostics recommends NT-proBNP results be interpreted
as in the table below(2):

Result.....Age.....Interpretation
<125.....All.....Negative: Heart Failure Unlikely
>=125.....All.....Consider Heart Failure as well as other causes* of NT-proBNP elevation.

* Natriuretic peptides value elevations could also be caused by conditions other than heart failure such as acute
coronary syndrome, pulmonary embolism, pulmonary hypertension, sepsis, stroke, and renal dysfunction (2).

References:
1. Terminal Pro-B-Type Natriuretic Peptide in the Emergency Department: The ICON-RELOADED Study. J Am Coll Cardiol. 2018 Mar 20;71(11):1191-1200. doi: 10.1016/j.jacc.2018.01.021. PMID: 29544601.

2. NBNP2 Instructions For Use. Ortho Clinical Diagnostics

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	6.4	3.4 - 11.0 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
RBC	3.16	3.98 - 5.25 M/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
HGB	9.5	11.9 - 15.3 g/dL		12/24/2024 1:05 AM PST	MERCY LAB CV	
HCT	31.7	37.3 - 46.7 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		12/24/2024 1:05 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		12/24/2024 1:05 AM PST	MERCY LAB CV	
MCHC	30.0	31.0 - 36.0 g/dL		12/24/2024 1:05 AM PST	MERCY LAB CV	
RDW	14.6	11.7 - 14.9 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Platelet Count	411	150 - 425 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
MPV	8.1	9.0 - 12.8 fL		12/24/2024 1:05 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		12/24/2024 1:05 AM PST	MERCY LAB CV	
Neutrophils %	52.3	38.0 - 74.0 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
IG%	4.1	0.0 - 1.2 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Lymphocytes %	31.8	16.0 - 48.0 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Monocytes %	6.3	4.9 - 12.5 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Eosinophils %	4.9	0.0 - 9.5 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Basophils %	0.6	0.0 - 1.6 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.32	1.50 - 7.40 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
IG#	0.26	0.00 - 0.10 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.02	0.90 - 3.10 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Monocytes	0.40	0.26 - 0.87 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.31	0.00 - 0.51 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Preliminary Abs Neut	3.32	1.50 - 7.40 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.041	<=0.034 ng/mL		12/24/2024 1:31 AM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population.  This patient's result is suspicious for myocardial injury. However,
serial measurements may be necessary to confirm or exclude the diagnosis of acute myocardial infarction.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/24/2024 1:18 AM PST	MERCY LAB CV	
Glucose	91	70 - 125 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	
Sodium	141	137 - 145 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Urea Nitrogen	36	7 - 17 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	
Creatinine	1.25	0.50 - 1.00 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	49	>60 mL/min/1.73m*2		12/24/2024 1:18 AM PST	MERCY LAB CV	
Anion Gap	8	6 - 14 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Osmolality Calculated	300	280 - 305 mOsm/kg H20		12/24/2024 1:18 AM PST	MERCY LAB CV	
Calcium	9.3	8.4 - 10.2 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

MERCY LAB CV - 12/24/2024 1:18 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (12/24/2024 12:50 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	86	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	86	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	178	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	100	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	410	ms			SCRIPPS RADIOLOGY	
QTC	490	ms			SCRIPPS RADIOLOGY	
P AXIS	59	degrees			SCRIPPS RADIOLOGY	
R AXIS	16	degrees			SCRIPPS RADIOLOGY	
T AXIS	91	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/24/2024 12:50 AM PST	12/24/2024 10:12 AM PST

Impressions

SCRIPPS RADIOLOGY - 12/24/2024 10:12 AM PST
Normal sinus rhythm with sinus arrhythmia
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific ST and T wave abnormality
Prolonged QT
Abnormal ECG
When compared with ECG of 23-Sep-2024 10:49,
aberrant conduction is no longer present

Narrative

This result has an attachment that is not available.

Procedure Note

Douglas Ballesteros, MD - 12/24/2024
Formatting of this note might be different from the original.
IMPRESSION:
Normal sinus rhythm with sinus arrhythmia
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific ST and T wave abnormality
Prolonged QT
Abnormal ECG
When compared with ECG of 23-Sep-2024 10:49,
aberrant conduction is no longer present

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

CULTURE, BLOOD (BLOOD CULTURE) - Final result (09/23/2024 4:19 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			09/28/2024 5:07 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of left upper limb / Unknown	Venipuncture / Unknown	09/23/2024 4:19 PM PDT	09/23/2024 4:21 PM PDT

Narrative

MERCY LAB CV - 09/28/2024 5:07 PM PDT
Reference range: No growth

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (09/23/2024 4:16 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			09/28/2024 5:07 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of left upper limb / Unknown	Venipuncture / Unknown	09/23/2024 4:16 PM PDT	09/23/2024 4:21 PM PDT

Narrative

MERCY LAB CV - 09/28/2024 5:07 PM PDT
Reference range: No growth

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray chest 2 views with pa/ap and lateral (X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL) - Final result (09/23/2024 11:47 AM PDT)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			09/23/2024 11:48 AM PDT	

Impressions

09/23/2024 11:49 AM PDT
IMPRESSION: Decreased right pleural effusion in comparison to 1/29/2024. No new infiltrate.

release

Narrative

09/23/2024 11:49 AM PDT

Narrative

EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 9/23/2024 11:47 AM

HISTORY: productive cough

COMPARISON: 1/29/2024

FINDINGS:

The cardiomediastinal silhouette is normal in size and contour.

Decreased right pleural effusion in comparison to 1/29/2024.

No evidence of fracture or suspicious osseous lesion.

Procedure Note

Brian Shore, MD - 09/23/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 9/23/2024 11:47 AM

HISTORY: productive cough

COMPARISON: 1/29/2024

FINDINGS:

The cardiomediastinal silhouette is normal in size and contour.

Decreased right pleural effusion in comparison to 1/29/2024.

No evidence of fracture or suspicious osseous lesion.

IMPRESSION:
IMPRESSION: Decreased right pleural effusion in comparison to 1/29/2024. No new infiltrate.

release

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	3,460	Refer to the reference range comment. pg/mL		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

NT-proBNP reference range: <=125 pg/mL

NT-proBNP testing aids in the diagnosis or exclusion of heart failure (HF) when used in conjunction with clinical history, symptoms, exam findings, and additional imaging and laboratory studies.

Emergency Department:
The ICON-RELOADED study provided validation of cutoff values for NT-proBNP in patients presenting with dyspnea in the Emergency Department (1).
In this setting NT-proBNP level <300 pg/mL has a high negative predictive value for HF. For values above 300 pg/mL, age-specific cutoffs provide additional decision limit categories:

Result.....Age.....Interpretation
<300.....All.....Negative: Heart Failure Unlikely
=300 to <450.....22-<50..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <900.....50-<75..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <1800....>=75....Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=>450.....22-<50..Positive: Heart Failure Likely
=>900.....50-<75..Positive: Heart Failure Likely
=>1800.....>=75....Positive: Heart Failure Likely

* Natriuretic peptides values in the gray zones may be caused by conditions other than heart failure. Acute coronary syndrome,pulmonary embolism,pulmonary hypertension,sepsis,stroke,and renal dysfunction will elevate NT- proBNP levels. Obesity, flash pulmonary edema,cardiac tamponade,and pericardial constriction are conditions associated with reduced NT-proBNP(2).

Ambulatory Clinics:
For patients presenting to outpatient clinics with suspicion of HF, the optimal clinical utility of NT-proBNP is to
exclude heart failure.For this clinical setting Ortho Clinical Diagnostics recommends NT-proBNP results be interpreted
as in the table below(2):

Result.....Age.....Interpretation
<125.....All.....Negative: Heart Failure Unlikely
>=125.....All.....Consider Heart Failure as well as other causes* of NT-proBNP elevation.

* Natriuretic peptides value elevations could also be caused by conditions other than heart failure such as acute
coronary syndrome, pulmonary embolism, pulmonary hypertension, sepsis, stroke, and renal dysfunction (2).

References:
1. Terminal Pro-B-Type Natriuretic Peptide in the Emergency Department: The ICON-RELOADED Study. J Am Coll Cardiol. 2018 Mar 20;71(11):1191-1200. doi: 10.1016/j.jacc.2018.01.021. PMID: 29544601.

2. NBNP2 Instructions For Use. Ortho Clinical Diagnostics

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROCALCITONIN - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Procalcitonin	0.36	0.03 - 0.08 ng/mL		09/23/2024 12:05 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 12:05 PM PDT
Procalcitonin (PCT) is a dynamic biomarker with value in guiding antibiotic management in select clinical settings. PCT values are most useful when trends are analyzed. Decisions on antibiotic use should not be based solely on PCT level. Consider repeating PCT no more than daily in ICU/sepsis patients, and every 2 days for LRTI in order to discontinue antibiotics.

Suspected Lower Respiratory Tract Infection (LRTI):
0.1-0.25 ng/mL - low likelihood for bacterial infection, antibiotics discouraged
>0.25 ng/mL - increased likelihood for bacterial infection.

Suspected Sepsis
0.1-0.5 ng/mL - low likelihood for sepsis, antibiotics discouraged.
>0.5 ng/mL - increased likelihood of sepsis.
>2.0 ng/mL - high risk of sepsis/septic shock.

Consider discontinuation of antibiotics when PCT <0.25 (LRTI) or <0.5 (sepsis), or 80% reduction from baseline.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	8.8	3.4 - 11.0 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
RBC	3.96	3.98 - 5.25 M/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
HGB	11.5	11.9 - 15.3 g/dL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
HCT	37.7	37.3 - 46.7 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MCV	95	81 - 100 fL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MCH	29	26 - 33 pg		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MCHC	30.5	31.0 - 36.0 g/dL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
RDW	14.4	11.7 - 14.9 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	336	150 - 425 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MPV	8.7	9.0 - 12.8 fL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Neutrophils %	78.4	38.0 - 74.0 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
IG%	0.3	0.0 - 1.2 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Lymphocytes %	16.9	16.0 - 48.0 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Monocytes %	4.2	4.9 - 12.5 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Eosinophils %	0.0	0.0 - 9.5 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Neutrophils	6.89	1.50 - 7.40 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Lymphocytes	1.49	0.90 - 3.10 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Monocytes	0.37	0.26 - 0.87 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Eosinophils	0.00	0.00 - 0.51 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Basophils	0.02	0.00 - 0.09 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Preliminary Abs Neut	6.89	1.50 - 7.40 K/ mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	<0.012	<=0.034 ng/mL		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population. This patient's result is considered normal. However, clinical
correlation is suggested to determine whether serial measurements are necessary for further evaluation.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

APTT (ACTIVATED PTT) - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
aPTT	36	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:25 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTIME-INR (PROTHROMBIN TIME-INR) - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protime	12.6	10.0 - 13.1 Seconds		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
INR	1.1	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:25 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	11.9	<10.0 mg/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				09/23/2024 11:54 AM PDT	MERCY LAB CV	
Glucose	118	70 - 125 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	102	98 - 107 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Urea Nitrogen	23	7 - 17 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Creatinine	0.81	0.50 - 1.00 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
CKD-EPI eGFR 2021	83	>60 mL/min/1.73m*2		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Anion Gap	7	6 - 14 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Osmolality Calculated	291	280 - 305 mOsm/kg H20		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Calcium	8.9	8.4 - 10.2 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Protein, total	7.9	6.3 - 8.2 g/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Alkaline Phosphatase	86	38 - 126 Units/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
AST	29	14 - 36 Units/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
ALT	15	<=34 Units/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Bilirubin, Total	0.4	0.2 - 1.3 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 11:54 AM PDT

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COVID-19, RAPID COMBO (COVID-19, RAPID (PCR/NAAT, COMBO)) - Final result (09/23/2024 11:16 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected		09/23/2024 12:32 PM PDT	MERCY LAB CV	

Comment:

A Detected result is considered a positive result for that virus.
A positive SARS-CoV-2 result indicates that RNA from SARS-CoV-2 was detected and that the patient is considered infected and presumed contagious. A positive Influenza A or B result indicates that RNA from Influenza A or B, respectively, was detected and that the patient is considered infected and presumed contagious.

A Not Detected (negative) test result means that RNA was not present in the specimen above the limit of detection. However, it does not rule out the possibility of infection and should not be used as the sole basis for patient management decisions. Test parameters for asymptomatic patients have not been evaluated.

These results have been reported to the San Diego Dept. of Health Services as required by Title 17, California Code of regulations, Section 2500.
Test information for patients can be found at the following link: https://www.fda.gov/media/142192/download
Test information for providers can be found at the following link: https://www.fda.gov/media/142191/download

This SARS-CoV-2 Real Time RT-PCR test was performed under an Emergency Use Authorization granted by the FDA for the Roche Liat.

This test meets Covid-19 testing requirements for school and business testing criteria and is not an antibody or antigen test.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		09/23/2024 11:16 AM PDT	09/23/2024 11:23 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

INFLUENZA A/B COMBO - Final result (09/23/2024 11:16 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Influenza A PCR	Not Detected	Not Detected		09/23/2024 12:32 PM PDT	MERCY LAB CV	
Influenza B PCR	Not Detected	Not Detected		09/23/2024 12:32 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		09/23/2024 11:16 AM PDT	09/23/2024 11:23 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 12:32 PM PDT
These results have been reported to the San Diego Department of Health Services as required by Title 17, California Code of Regulations, Section 2505.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RSV BY PCR (RSV (PCR/NAAT)) - Final result (09/23/2024 11:15 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
RSV by PCR	Not Detected	Not Detected		09/23/2024 12:33 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
NP Swab	Nasopharyngeal structure / Unknown		09/23/2024 11:15 AM PDT	09/23/2024 11:23 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 12:33 PM PDT
These results have been reported to the San Diego Department of Health Services as required by Title 17, California Code of regulations, Section 2505.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (09/23/2024 10:49 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	101	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	101	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	158	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	92	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	370	ms			SCRIPPS RADIOLOGY	
QTC	479	ms			SCRIPPS RADIOLOGY	
P AXIS	60	degrees			SCRIPPS RADIOLOGY	
R AXIS	14	degrees			SCRIPPS RADIOLOGY	
T AXIS	123	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			09/23/2024 10:49 AM PDT	09/23/2024 2:20 PM PDT

Impressions

SCRIPPS RADIOLOGY - 09/23/2024 2:20 PM PDT
Sinus tachycardia with premature atrial complexes with aberrant conduction
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific T wave abnormality
Abnormal ECG
When compared with ECG of 11-Feb-2024 10:50,
aberrant conduction is now present
Nonspecific T wave abnormality no longer evident in Inferior leads
T wave inversion less evident in Lateral leads

Narrative

This result has an attachment that is not available.

Procedure Note

Mark Hernandez, MD - 09/23/2024
Formatting of this note might be different from the original.
IMPRESSION:
Sinus tachycardia with premature atrial complexes with aberrant conduction
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific T wave abnormality
Abnormal ECG
When compared with ECG of 11-Feb-2024 10:50,
aberrant conduction is now present
Nonspecific T wave abnormality no longer evident in Inferior leads
T wave inversion less evident in Lateral leads

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

Echocardiogram 2D With Definity Protocol As Needed (ECHOCARDIOGRAM 2D WITH IMAGE ENHANCEMENT AGENT AS NEEDED) - Final result (09/18/2024 2:54 PM PDT)

Anatomical Region	Laterality	Modality
		Ultrasound

Narrative

This result has an attachment that is not available.

Authorizing Provider	Result Type	Result Status
Jerrold Glassman MD	CV ECHO PROCEDURES	Final Result

12-LEAD ECG - Final result (08/14/2024 4:20 PM PDT)

Narrative

This result has an attachment that is not available.

Authorizing Provider	Result Type	Result Status
Jerrold Glassman MD	ECG ORDERABLES	Final Result

Nuc Med hepatobiliary sys without pharm including gallbladder (NUC MED HEPATOBILIARY SYSTEM WITHOUT PHARMACOLOGY) - Final result (02/16/2024 3:20 PM PST)

Anatomical Region	Laterality	Modality
		Nuclear Medicine

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/16/2024 3:16 PM PST	

Impressions

02/16/2024 3:59 PM PST

IMPRESSION: Normal gallbladder visualization. There is no evidence of acute cholecystitis or biliary ductal dilation.

release

Narrative

02/16/2024 3:59 PM PST

EXAMINATION: NUC MED HEPATOBILIARY SYSTEM WITHOUT PHARMACOLOGY

DATE/TIME: 2/16/2024 1:55 PM

HISTORY: RUQ abdominal pain, biliary disease suspected, US nondiagnostic

COMPARISON: Right upper quadrant ultrasound 2/15/2024

TECHNIQUE: Radiopharmaceutical was injected intravenously and sequential static biliary tract imaging was performed.

PHARMACEUTICAL: 5.3 mCi Tc-99m biliary IDA derivative

FINDINGS:
BILIARY DUCTS: Normal radioisotopic biliary excretion.
GALLBLADDER: Normal gallbladder visualization beginning at 25 minutes with no evidence of cystic duct obstruction.
INTESTINE: Normal progression of tracer into the small bowel seen beginning at 35 minutes. No evidence of common biliary ductal obstruction.
OTHER: None.

Procedure Note

Lida Chaipat, MD - 02/16/2024

Formatting of this note might be different from the original.

EXAMINATION: NUC MED HEPATOBILIARY SYSTEM WITHOUT PHARMACOLOGY

DATE/TIME: 2/16/2024 1:55 PM

HISTORY: RUQ abdominal pain, biliary disease suspected, US nondiagnostic

COMPARISON: Right upper quadrant ultrasound 2/15/2024

TECHNIQUE: Radiopharmaceutical was injected intravenously and sequential static biliary tract imaging was performed.

PHARMACEUTICAL: 5.3 mCi Tc-99m biliary IDA derivative

FINDINGS:
BILIARY DUCTS: Normal radioisotopic biliary excretion.
GALLBLADDER: Normal gallbladder visualization beginning at 25 minutes with no evidence of cystic duct obstruction.
INTESTINE: Normal progression of tracer into the small bowel seen beginning at 35 minutes. No evidence of common biliary ductal obstruction.
OTHER: None.

IMPRESSION:
IMPRESSION: Normal gallbladder visualization. There is no evidence of acute cholecystitis or biliary ductal dilation.

release

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	IMG NM PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (02/16/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.0	3.4 - 11.0 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
RBC	3.21	3.98 - 5.25 M/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
HGB	9.7	11.9 - 15.3 g/dL		02/16/2024 6:36 AM PST	MERCY LAB CV	
HCT	32.3	37.3 - 46.7 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/16/2024 6:36 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		02/16/2024 6:36 AM PST	MERCY LAB CV	
MCHC	30.0	31.0 - 36.0 g/dL		02/16/2024 6:36 AM PST	MERCY LAB CV	
RDW	13.4	11.7 - 14.9 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Platelet Count	305	150 - 425 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
MPV	8.4	9.0 - 12.8 fL		02/16/2024 6:36 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/16/2024 6:36 AM PST	MERCY LAB CV	
Neutrophils %	38.2	38.0 - 74.0 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
IG%	0.2	0.0 - 1.2 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Lymphocytes %	45.0	16.0 - 48.0 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Monocytes %	8.2	4.9 - 12.5 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Eosinophils %	7.4	0.0 - 9.5 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Neutrophils	1.91	1.50 - 7.40 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
IG#	0.01	0.00 - 0.10 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.25	0.90 - 3.10 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Monocytes	0.41	0.26 - 0.87 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.37	0.00 - 0.51 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Preliminary Abs Neut	1.91	1.50 - 7.40 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/16/2024 5:45 AM PST	02/16/2024 6:02 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/16/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/16/2024 6:24 AM PST	MERCY LAB CV	
Glucose	113	70 - 125 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Sodium	137	137 - 145 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Urea Nitrogen	23	7 - 17 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		02/16/2024 6:24 AM PST	MERCY LAB CV	
Anion Gap	6	6 - 14 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality Calculated	288	280 - 305 mOsm/kg H20		02/16/2024 6:24 AM PST	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Protein, total	8.4	6.3 - 8.2 g/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Alkaline Phosphatase	80	38 - 126 Units/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
AST	38	14 - 36 Units/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
ALT	26	<=34 Units/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/16/2024 5:45 AM PST	02/16/2024 6:02 AM PST

Narrative

MERCY LAB CV - 02/16/2024 6:24 AM PST

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for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

Ultrasound abdominal limited gallbladder (ULTRASOUND ABDOMEN LIMITED GALLBLADDER) - Final result (02/15/2024 6:30 PM PST)

Anatomical Region	Laterality	Modality
Body		Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/15/2024 6:58 PM PST	

Impressions

02/15/2024 7:01 PM PST

Impressions

IMPRESSION:
1. Tumefactive sludge favored over nonshadowing new gallstones compared to negative gallbladder study of 1/13/2024.
2. Slight gallbladder wall thickening without pericholecystic edema or sonographic Murphy sign not so pronounced to confirm acute cholecystitis. Nuclear medicine biliary scan can clarify if additional imaging indicated clinically.
3. Borderline hepatomegaly with slightly echogenic dense sonographic pattern suspicious for mild hepatic steatosis. No surface contour abnormality, or ascites to suggest cirrhosis. No visible lesion to suggest infiltrative malignancy.

release

Narrative

02/15/2024 7:01 PM PST
EXAMINATION: ULTRASOUND ABDOMEN LIMITED GALLBLADDER

DATE/TIME: 2/15/2024 6:30 PM

HISTORY: RUQ pain

ADDITIONAL CLINICAL INFORMATION: CHF.

COMPARISON: 1/13/2024.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: The echogenicity of the liver is increased. Liver is mildly enlarged at 17.4 cm cephalocaudad reaching well below the lower pole the right kidney. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
GALLBLADDER: The gallbladder now contains echogenic debris, nonshadowing. This most likely is related to tumefactive sludge rather than nonshadowing gallstones. Filled biliary interval dilatation of the common bile duct to 8.2 mm without visible calcified common duct stone or mass. No intra paddock duct dilatation.
BILIARY: No ductal dilatation.
PANCREAS: Visualized pancreatic head and body negative for focal mass or ductal dilatation.
RIGHT KIDNEY: No visible solid mass, calculus or hydronephrosis.
AORTA/VASC: No visible aneurysm.
IVC: The visible IVC is patent.
OTHER: No free fluid.

The following measurements were obtained:
AORTA PROXIMAL: 2.1 cm
AORTA DISTAL: 1.5 cm
LIVER: 17.4 cm
GALLBLADDER WALL: 3.2 mm
CBD: 8.2 mm
RT KIDNEY: 10.4 cm x 4.4 cm x 5.3 cm

Procedure Note

Kris Vanlom, MD - 02/15/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: ULTRASOUND ABDOMEN LIMITED GALLBLADDER

DATE/TIME: 2/15/2024 6:30 PM

HISTORY: RUQ pain

ADDITIONAL CLINICAL INFORMATION: CHF.

COMPARISON: 1/13/2024.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: The echogenicity of the liver is increased. Liver is mildly enlarged at 17.4 cm cephalocaudad reaching well below the lower pole the right kidney. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
GALLBLADDER: The gallbladder now contains echogenic debris, nonshadowing. This most likely is related to tumefactive sludge rather than nonshadowing gallstones. Filled biliary interval dilatation of the common bile duct to 8.2 mm without visible calcified common duct stone or mass. No intra paddock duct dilatation.
BILIARY: No ductal dilatation.
PANCREAS: Visualized pancreatic head and body negative for focal mass or ductal dilatation.
RIGHT KIDNEY: No visible solid mass, calculus or hydronephrosis.
AORTA/VASC: No visible aneurysm.
IVC: The visible IVC is patent.
OTHER: No free fluid.

The following measurements were obtained:
AORTA PROXIMAL: 2.1 cm
AORTA DISTAL: 1.5 cm
LIVER: 17.4 cm
GALLBLADDER WALL: 3.2 mm
CBD: 8.2 mm
RT KIDNEY: 10.4 cm x 4.4 cm x 5.3 cm

IMPRESSION:
IMPRESSION:
1. Tumefactive sludge favored over nonshadowing new gallstones compared to negative gallbladder study of 1/13/2024.
2. Slight gallbladder wall thickening without pericholecystic edema or sonographic Murphy sign not so pronounced to confirm acute cholecystitis. Nuclear medicine biliary scan can clarify if additional imaging indicated clinically.
3. Borderline hepatomegaly with slightly echogenic dense sonographic pattern suspicious for mild hepatic steatosis. No surface contour abnormality, or ascites to suggest cirrhosis. No visible lesion to suggest infiltrative malignancy.

release

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	IMG US PROCEDURES	Final Result

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (02/14/2024 11:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	111	<=30 mm/hr		02/14/2024 1:44 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/14/2024 11:11 AM PST	02/14/2024 11:27 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (02/14/2024 11:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	<5.0	<10.0 mg/L		02/14/2024 11:45 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/14/2024 11:11 AM PST	02/14/2024 11:24 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/14/2024 5:47 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	104	70 - 125 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Sodium	137	137 - 145 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
CO2, Total	33	22 - 30 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Creatinine	0.96	0.50 - 1.00 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	68	>60 mL/min/1.73m*2		02/14/2024 6:24 AM PST	MERCY LAB CV	
Anion Gap	3	6 - 14 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality Calculated	287	280 - 305 mOsm/kg H20		02/14/2024 6:24 AM PST	MERCY LAB CV	
Calcium	8.9	8.4 - 10.2 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Phosphorus	5.6	2.5 - 4.5 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Albumin	3.4	3.5 - 5.0 g/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/14/2024 5:47 AM PST	02/14/2024 6:00 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/13/2024 7:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
RBC	3.20	3.98 - 5.25 M/mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
HGB	9.8	11.9 - 15.3 g/dL		02/13/2024 8:01 AM PST	MERCY LAB CV	
HCT	32.2	37.3 - 46.7 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/13/2024 8:01 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/13/2024 8:01 AM PST	MERCY LAB CV	
MCHC	30.4	31.0 - 36.0 g/dL		02/13/2024 8:01 AM PST	MERCY LAB CV	
RDW	13.8	11.7 - 14.9 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Platelet Count	333	150 - 425 K/mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
MPV	8.3	9.0 - 12.8 fL		02/13/2024 8:01 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
nRBC	0.4	0.0 /100 WBCs		02/13/2024 8:01 AM PST	MERCY LAB CV	
Neutrophils %	44.1	38.0 - 74.0 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
IG%	0.4	0.0 - 1.2 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Lymphocytes %	40.9	16.0 - 48.0 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Monocytes %	8.9	4.9 - 12.5 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Eosinophils %	4.9	0.0 - 9.5 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.35	1.50 - 7.40 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.17	0.90 - 3.10 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Monocytes	0.47	0.26 - 0.87 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.26	0.00 - 0.51 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.35	1.50 - 7.40 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/13/2024 7:36 AM PST	02/13/2024 7:40 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/13/2024 7:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	95	70 - 125 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Sodium	139	137 - 145 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Urea Nitrogen	22	7 - 17 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		02/13/2024 8:03 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Osmolality Calculated	291	280 - 305 mOsm/kg H20		02/13/2024 8:03 AM PST	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Phosphorus	5.1	2.5 - 4.5 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/13/2024 7:36 AM PST	02/13/2024 7:40 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/13/2024 7:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/13/2024 8:03 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	95	70 - 125 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Sodium	139	137 - 145 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Urea Nitrogen	22	7 - 17 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		02/13/2024 8:03 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Osmolality Calculated	291	280 - 305 mOsm/kg H20		02/13/2024 8:03 AM PST	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Protein, total	8.4	6.3 - 8.2 g/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Alkaline Phosphatase	79	38 - 126 Units/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
AST	35	14 - 36 Units/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
ALT	25	<=34 Units/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/13/2024 7:36 AM PST	02/13/2024 7:40 AM PST

Narrative

MERCY LAB CV - 02/13/2024 8:03 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/12/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	97	70 - 125 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Urea Nitrogen	21	7 - 17 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Creatinine	0.81	0.50 - 1.00 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	84	>60 mL/min/1.73m*2		02/12/2024 6:37 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Osmolality Calculated	289	280 - 305 mOsm/kg H20		02/12/2024 6:37 AM PST	MERCY LAB CV	
Calcium	9.0	8.4 - 10.2 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Phosphorus	5.3	2.5 - 4.5 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Albumin	3.5	3.5 - 5.0 g/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/12/2024 5:28 AM PST	02/12/2024 6:12 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/11/2024 8:09 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	113	70 - 125 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Urea Nitrogen	23	7 - 17 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Creatinine	0.84	0.50 - 1.00 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	80	>60 mL/min/1.73m*2		02/11/2024 8:46 PM PST	MERCY LAB CV	
Anion Gap	4	6 - 14 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Osmolality Calculated	290	280 - 305 mOsm/kg H20		02/11/2024 8:46 PM PST	MERCY LAB CV	
Calcium	8.8	8.4 - 10.2 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Phosphorus	4.3	2.5 - 4.5 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Albumin	3.6	3.5 - 5.0 g/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/11/2024 8:09 PM PST	02/11/2024 8:20 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray hip with and without pelvis right 2-3 views (X-RAY HIP RIGHT 2-3 VIEWS) - Final result (02/11/2024 4:26 PM PST)

Anatomical Region	Laterality	Modality
Hip	Right	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/11/2024 4:42 PM PST	

Impressions

02/12/2024 11:04 AM PST
IMPRESSION:
1. Mild degenerative changes at right hip joint. No acute fracture detected.

release

Narrative

02/12/2024 11:04 AM PST
EXAMINATION: X-RAY HIP RIGHT 2-3 VIEWS

DATE/TIME: 2/11/2024 4:26 PM

HISTORY: Right hip pain

COMPARISON: None.

FINDINGS:

Two upright views of the right hip obtained.

There is normal alignment of the right hip joint without dislocation. There is mild superior joint space narrowing at the right hip joint. Minimal degenerative spurring seen. No plain film evident acute fracture at the right hip detected.

There is normal contour and density of the right femoral head.

Mild degenerative changes of the pubic symphysis seen.

Procedure Note

Jerome Pierce, MD - 02/12/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY HIP RIGHT 2-3 VIEWS

DATE/TIME: 2/11/2024 4:26 PM

HISTORY: Right hip pain

COMPARISON: None.

FINDINGS:

Two upright views of the right hip obtained.

There is normal alignment of the right hip joint without dislocation. There is mild superior joint space narrowing at the right hip joint. Minimal degenerative spurring seen. No plain film evident acute fracture at the right hip detected.

There is normal contour and density of the right femoral head.

Mild degenerative changes of the pubic symphysis seen.

IMPRESSION:
IMPRESSION:
1. Mild degenerative changes at right hip joint. No acute fracture detected.

release

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (02/11/2024 4:08 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/11/2024 4:11 PM PST	

Impressions

02/11/2024 4:11 PM PST
IMPRESSION: No deep venous thrombosis detected in either leg.

release

Narrative

02/11/2024 4:11 PM PST

Narrative

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 2/11/2024 4:08 PM

HISTORY: Dvt

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable. The technologist could not see the left peroneal vein possibly due to presence of edema.
OTHER: None.

Procedure Note

Jerome Pierce, MD - 02/11/2024

Formatting of this note might be different from the original.

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 2/11/2024 4:08 PM

HISTORY: Dvt

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable. The technologist could not see the left peroneal vein possibly due to presence of edema.
OTHER: None.

IMPRESSION:
IMPRESSION: No deep venous thrombosis detected in either leg.

release

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	IMG US PROCEDURES	Final Result

12-LEAD ECG - Final result (02/11/2024 10:50 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	98	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	98	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	184	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	90	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	386	ms			SCRIPPS RADIOLOGY	
QTC	492	ms			SCRIPPS RADIOLOGY	
P AXIS	25	degrees			SCRIPPS RADIOLOGY	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
R AXIS	55	degrees			SCRIPPS RADIOLOGY	
T AXIS	133	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/11/2024 10:50 AM PST	02/23/2024 3:34 PM PST

Impressions

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SCRIPPS RADIOLOGY - 02/23/2024 3:34 PM PST
Normal sinus rhythm
ST & T wave abnormality, consider lateral ischemia
Abnormal ECG
```

Narrative

This result has an attachment that is not available.

Procedure Note

Raghava Gollapudi, MD - 02/23/2024

Formatting of this note might be different from the original.

IMPRESSION:
Normal sinus rhythm
ST & T wave abnormality, consider lateral ischemia
Abnormal ECG

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

CBC WITH DIFFERENTIAL - Final result (02/10/2024 5:52 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	6.0	3.4 - 11.0 K/mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
RBC	3.25	3.98 - 5.25 M/mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
HGB	10.1	11.9 - 15.3 g/dL		02/10/2024 6:39 AM PST	MERCY LAB CV	
HCT	32.4	37.3 - 46.7 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/10/2024 6:39 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/10/2024 6:39 AM PST	MERCY LAB CV	
MCHC	31.2	31.0 - 36.0 g/dL		02/10/2024 6:39 AM PST	MERCY LAB CV	
RDW	13.8	11.7 - 14.9 %		02/10/2024 6:39 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	377	150 - 425 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
MPV	8.4	9.0 - 12.8 fL		02/10/2024 6:39 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/10/2024 6:39 AM PST	MERCY LAB CV	
Neutrophils %	45.9	38.0 - 74.0 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
IG%	0.3	0.0 - 1.2 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Lymphocytes %	41.3	16.0 - 48.0 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Monocytes %	8.2	4.9 - 12.5 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Eosinophils %	3.5	0.0 - 9.5 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.74	1.50 - 7.40 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.47	0.90 - 3.10 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Monocytes	0.49	0.26 - 0.87 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.21	0.00 - 0.51 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.74	1.50 - 7.40 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/10/2024 5:52 AM PST	02/10/2024 6:32 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/10/2024 5:52 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/10/2024 7:00 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Urea Nitrogen	21	7 - 17 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/10/2024 7:00 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Osmolality Calculated	277	280 - 305 mOsm/kg H20		02/10/2024 7:00 AM PST	MERCY LAB CV	
Calcium	9.0	8.4 - 10.2 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Protein, total	8.5	6.3 - 8.2 g/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Albumin	3.6	3.5 - 5.0 g/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Alkaline Phosphatase	80	38 - 126 Units/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
AST	33	14 - 36 Units/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
ALT	21	<=34 Units/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/10/2024 5:52 AM PST	02/10/2024 6:31 AM PST

Narrative

MERCY LAB CV - 02/10/2024 7:00 AM PST

```
for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (02/09/2024 10:58 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/09/2024 11:35 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	
Sodium	136	137 - 145 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Urea Nitrogen	22	7 - 17 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	
Creatinine	1.00	0.50 - 1.00 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	65	>60 mL/min/1.73m*2		02/09/2024 11:35 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Osmolality Calculated	285	280 - 305 mOsm/kg H20		02/09/2024 11:35 AM PST	MERCY LAB CV	
Calcium	8.9	8.4 - 10.2 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/09/2024 10:58 AM PST	02/09/2024 11:18 AM PST

Narrative

MERCY LAB CV - 02/09/2024 11:35 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray ankle left 3 or more views (X-RAY ANKLE LEFT 3 OR MORE VIEWS) - Final result (02/07/2024 5:27 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities, Ankle	Left	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/07/2024 5:56 PM PST	

Impressions

02/07/2024 5:57 PM PST
IMPRESSION:
1. Severe soft tissue swelling around the ankle midfoot and forefoot correlate clinically for cellulitis. No radiographic evidence of osteomyelitis. Midfoot degenerative changes

2. No soft tissue emphysema

release

Narrative

02/07/2024 5:57 PM PST
EXAMINATION: X-RAY ANKLE LEFT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle pain, no prior imaging

COMPARISON: None.

FINDINGS: See impression.

Procedure Note

George Wesbey, MD - 02/07/2024

Procedure Note

Formatting of this note might be different from the original.
EXAMINATION: X-RAY ANKLE LEFT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle pain, no prior imaging

COMPARISON: None.

FINDINGS: See impression.

IMPRESSION:
IMPRESSION:
1. Severe soft tissue swelling around the ankle midfoot and forefoot correlate clinically for cellulitis. No radiographic evidence of osteomyelitis. Midfoot degenerative changes

2. No soft tissue emphysema

release

Authorizing Provider	Result Type	Result Status
Howie Que DPM	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

X-ray ankle right 3 or more views (X-RAY ANKLE RIGHT 3 OR MORE VIEWS) - Final result (02/07/2024 5:27 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities, Ankle	Right	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/07/2024 5:57 PM PST	

Impressions

02/07/2024 5:58 PM PST
IMPRESSION:
1. Marked soft tissue swelling at the ankle and hindfoot and midfoot and forefoot

2. Small plantar calcaneal spur

3. No fractures or dislocations identified

release

Narrative

02/07/2024 5:58 PM PST
EXAMINATION: X-RAY ANKLE RIGHT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle trauma, fracture, xray done (Age >= 5y)

COMPARISON: None.

FINDINGS: See impression.

Procedure Note

George Wesbey, MD - 02/07/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY ANKLE RIGHT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle trauma, fracture, xray done (Age >= 5y)

COMPARISON: None.

FINDINGS: See impression.

IMPRESSION:
IMPRESSION:
1. Marked soft tissue swelling at the ankle and hindfoot and midfoot and forefoot

2. Small plantar calcaneal spur

3. No fractures or dislocations identified

release

Authorizing Provider	Result Type	Result Status
Howie Que DPM	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (02/07/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
RBC	2.96	3.98 - 5.25 M/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
HGB	9.2	11.9 - 15.3 g/dL		02/07/2024 6:51 AM PST	MERCY LAB CV	
HCT	29.5	37.3 - 46.7 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/07/2024 6:51 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/07/2024 6:51 AM PST	MERCY LAB CV	
MCHC	31.2	31.0 - 36.0 g/dL		02/07/2024 6:51 AM PST	MERCY LAB CV	
RDW	14.1	11.7 - 14.9 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Platelet Count	446	150 - 425 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
MPV	8.3	9.0 - 12.8 fL		02/07/2024 6:51 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/07/2024 6:51 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Neutrophils %	44.2	38.0 - 74.0 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Lymphocytes %	41.7	16.0 - 48.0 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Monocytes %	9.2	4.9 - 12.5 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Eosinophils %	3.2	0.0 - 9.5 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Basophils %	0.9	0.0 - 1.6 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.34	1.50 - 7.40 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.21	0.90 - 3.10 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Monocytes	0.49	0.26 - 0.87 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.17	0.00 - 0.51 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.34	1.50 - 7.40 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/07/2024 5:35 AM PST	02/07/2024 6:34 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/07/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/07/2024 7:34 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	94	70 - 125 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/07/2024 7:34 AM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Osmolality Calculated	274	280 - 305 mOsm/kg H20		02/07/2024 7:34 AM PST	MERCY LAB CV	
Calcium	8.8	8.4 - 10.2 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Protein, total	8.0	6.3 - 8.2 g/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Albumin	3.3	3.5 - 5.0 g/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Alkaline Phosphatase	90	38 - 126 Units/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
AST	32	14 - 36 Units/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
ALT	21	<=34 Units/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/07/2024 5:35 AM PST	02/07/2024 6:34 AM PST

Narrative

MERCY LAB CV - 02/07/2024 7:34 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/06/2024 5:29 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.0	3.4 - 11.0 K/mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
RBC	3.06	3.98 - 5.25 M/mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
HGB	9.1	11.9 - 15.3 g/dL		02/06/2024 6:32 AM PST	MERCY LAB CV	
HCT	30.5	37.3 - 46.7 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/06/2024 6:32 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		02/06/2024 6:32 AM PST	MERCY LAB CV	
MCHC	29.8	31.0 - 36.0 g/dL		02/06/2024 6:32 AM PST	MERCY LAB CV	
RDW	14.2	11.7 - 14.9 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Platelet Count	472	150 - 425 K/mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
MPV	8.2	9.0 - 12.8 fL		02/06/2024 6:32 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/06/2024 6:32 AM PST	MERCY LAB CV	
Neutrophils %	42.6	38.0 - 74.0 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
IG%	0.6	0.0 - 1.2 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Lymphocytes %	42.5	16.0 - 48.0 %		02/06/2024 6:32 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	10.5	4.9 - 12.5 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Eosinophils %	2.8	0.0 - 9.5 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.11	1.50 - 7.40 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.11	0.90 - 3.10 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Monocytes	0.52	0.26 - 0.87 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.14	0.00 - 0.51 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.11	1.50 - 7.40 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/06/2024 5:29 AM PST	02/06/2024 6:04 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/06/2024 5:29 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/06/2024 6:36 AM PST	MERCY LAB CV	
Glucose	103	70 - 125 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Potassium	4.1	3.5 - 5.1 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	103	98 - 107 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
CO2, Total	25	22 - 30 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Creatinine	0.71	0.50 - 1.00 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	98	>60 mL/min/1.73m*2		02/06/2024 6:36 AM PST	MERCY LAB CV	
Anion Gap	4	6 - 14 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Osmolality Calculated	275	280 - 305 mOsm/kg H20		02/06/2024 6:36 AM PST	MERCY LAB CV	
Calcium	8.8	8.4 - 10.2 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Protein, total	7.9	6.3 - 8.2 g/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Albumin	3.3	3.5 - 5.0 g/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Alkaline Phosphatase	84	38 - 126 Units/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
AST	36	14 - 36 Units/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
ALT	26	<=34 Units/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/06/2024 5:29 AM PST	02/06/2024 6:03 AM PST

Narrative

MERCY LAB CV - 02/06/2024 6:36 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/05/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
RBC	2.88	3.98 - 5.25 M/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
HGB	8.8	11.9 - 15.3 g/dL		02/05/2024 7:25 AM PST	MERCY LAB CV	
HCT	29.0	37.3 - 46.7 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/05/2024 7:25 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/05/2024 7:25 AM PST	MERCY LAB CV	
MCHC	30.3	31.0 - 36.0 g/dL		02/05/2024 7:25 AM PST	MERCY LAB CV	
RDW	13.9	11.7 - 14.9 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Platelet Count	515	150 - 425 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
MPV	8.1	9.0 - 12.8 fL		02/05/2024 7:25 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/05/2024 7:25 AM PST	MERCY LAB CV	
Neutrophils %	49.7	38.0 - 74.0 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Lymphocytes %	35.5	16.0 - 48.0 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Monocytes %	10.2	4.9 - 12.5 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Eosinophils %	3.0	0.0 - 9.5 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.64	1.50 - 7.40 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	1.88	0.90 - 3.10 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Monocytes	0.54	0.26 - 0.87 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.16	0.00 - 0.51 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.64	1.50 - 7.40 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/05/2024 5:53 AM PST	02/05/2024 6:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/05/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/05/2024 7:10 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Chloride	98	98 - 107 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Urea Nitrogen	15	7 - 17 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Creatinine	0.75	0.50 - 1.00 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	92	>60 mL/min/1.73m*2		02/05/2024 7:10 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Anion Gap	4	6 - 14 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Osmolality Calculated	273	280 - 305 mOsm/kg H20		02/05/2024 7:10 AM PST	MERCY LAB CV	
Calcium	8.7	8.4 - 10.2 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Protein, total	8.0	6.3 - 8.2 g/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Albumin	3.2	3.5 - 5.0 g/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Alkaline Phosphatase	107	38 - 126 Units/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
AST	39	14 - 36 Units/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
ALT	22	<=34 Units/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/05/2024 5:53 AM PST	02/05/2024 6:43 AM PST

Narrative

MERCY LAB CV - 02/05/2024 7:10 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/04/2024 4:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.1	3.4 - 11.0 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
RBC	2.67	3.98 - 5.25 M/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HGB	8.0	11.9 - 15.3 g/dL		02/04/2024 5:35 AM PST	MERCY LAB CV	
HCT	27.2	37.3 - 46.7 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		02/04/2024 5:35 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		02/04/2024 5:35 AM PST	MERCY LAB CV	
MCHC	29.4	31.0 - 36.0 g/dL		02/04/2024 5:35 AM PST	MERCY LAB CV	
RDW	13.9	11.7 - 14.9 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Platelet Count	461	150 - 425 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		02/04/2024 5:35 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/04/2024 5:35 AM PST	MERCY LAB CV	
Neutrophils %	44.3	38.0 - 74.0 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
IG%	1.2	0.0 - 1.2 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Lymphocytes %	38.5	16.0 - 48.0 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Monocytes %	10.8	4.9 - 12.5 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Eosinophils %	4.2	0.0 - 9.5 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Neutrophils	1.81	1.50 - 7.40 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
IG#	0.05	0.00 - 0.10 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.57	0.90 - 3.10 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Monocytes	0.44	0.26 - 0.87 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.17	0.00 - 0.51 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Preliminary Abs Neut	1.81	1.50 - 7.40 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/04/2024 4:11 AM PST	02/04/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/04/2024 4:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/04/2024 6:47 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Potassium	3.9	3.5 - 5.1 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/04/2024 6:47 AM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Osmolality Calculated	273	280 - 305 mOsm/kg H20		02/04/2024 6:47 AM PST	MERCY LAB CV	
Calcium	8.7	8.4 - 10.2 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protein, total	7.3	6.3 - 8.2 g/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Albumin	2.9	3.5 - 5.0 g/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Alkaline Phosphatase	81	38 - 126 Units/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
AST	38	14 - 36 Units/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
ALT	22	<=34 Units/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Bilirubin, Total	<0.1	0.2 - 1.3 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/04/2024 4:11 AM PST	02/04/2024 5:24 AM PST

Narrative

MERCY LAB CV - 02/04/2024 6:47 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/03/2024 5:42 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.5	3.4 - 11.0 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
RBC	2.62	3.98 - 5.25 M/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
HGB	8.0	11.9 - 15.3 g/dL		02/03/2024 6:07 AM PST	MERCY LAB CV	
HCT	26.4	37.3 - 46.7 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/03/2024 6:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCH	31	26 - 33 pg		02/03/2024 6:07 AM PST	MERCY LAB CV	
MCHC	30.3	31.0 - 36.0 g/dL		02/03/2024 6:07 AM PST	MERCY LAB CV	
RDW	14.1	11.7 - 14.9 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Platelet Count	496	150 - 425 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
MPV	7.7	9.0 - 12.8 fL		02/03/2024 6:07 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/03/2024 6:07 AM PST	MERCY LAB CV	
Neutrophils %	52.8	38.0 - 74.0 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
IG%	1.3	0.0 - 1.2 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Lymphocytes %	32.7	16.0 - 48.0 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Monocytes %	8.5	4.9 - 12.5 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Eosinophils %	4.0	0.0 - 9.5 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.36	1.50 - 7.40 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
IG#	0.06	0.00 - 0.10 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.46	0.90 - 3.10 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.38	0.26 - 0.87 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.18	0.00 - 0.51 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.36	1.50 - 7.40 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/03/2024 5:42 AM PST	02/03/2024 5:48 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/03/2024 5:42 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/03/2024 6:56 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Urea Nitrogen	19	7 - 17 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Creatinine	0.82	0.50 - 1.00 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	82	>60 mL/min/1.73m*2		02/03/2024 6:56 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Osmolality Calculated	274	280 - 305 mOsm/kg H20		02/03/2024 6:56 AM PST	MERCY LAB CV	
Calcium	8.6	8.4 - 10.2 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Protein, total	7.5	6.3 - 8.2 g/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Albumin	3.0	3.5 - 5.0 g/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Alkaline Phosphatase	76	38 - 126 Units/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
AST	42	14 - 36 Units/L		02/03/2024 6:56 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ALT	24	<=34 Units/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/03/2024 5:42 AM PST	02/03/2024 5:48 AM PST

Narrative

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MERCY LAB CV - 02/03/2024 6:56 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/02/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
RBC	2.59	3.98 - 5.25 M/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
HGB	7.9	11.9 - 15.3 g/dL		02/02/2024 5:47 AM PST	MERCY LAB CV	
HCT	26.2	37.3 - 46.7 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/02/2024 5:47 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/02/2024 5:47 AM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		02/02/2024 5:47 AM PST	MERCY LAB CV	
RDW	13.9	11.7 - 14.9 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Platelet Count	513	150 - 425 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MPV	7.8	9.0 - 12.8 fL		02/02/2024 5:47 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/02/2024 5:47 AM PST	MERCY LAB CV	
Neutrophils %	44.9	38.0 - 74.0 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
IG%	1.1	0.0 - 1.2 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Lymphocytes %	39.9	16.0 - 48.0 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Monocytes %	8.6	4.9 - 12.5 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Eosinophils %	4.4	0.0 - 9.5 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Basophils %	1.1	0.0 - 1.6 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.36	1.50 - 7.40 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
IG#	0.06	0.00 - 0.10 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.10	0.90 - 3.10 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Monocytes	0.45	0.26 - 0.87 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.23	0.00 - 0.51 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Basophils	0.06	0.00 - 0.09 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.36	1.50 - 7.40 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/02/2024 5:21 AM PST	02/02/2024 5:34 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/02/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/02/2024 5:50 AM PST	MERCY LAB CV	
Glucose	88	70 - 125 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Sodium	130	137 - 145 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Urea Nitrogen	19	7 - 17 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/02/2024 5:50 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Osmolality Calculated	272	280 - 305 mOsm/kg H20		02/02/2024 5:50 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Protein, total	7.2	6.3 - 8.2 g/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Alkaline Phosphatase	71	38 - 126 Units/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
AST	28	14 - 36 Units/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Bilirubin, Total	<0.1	0.2 - 1.3 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/02/2024 5:21 AM PST	02/02/2024 5:33 AM PST

Narrative

MERCY LAB CV - 02/02/2024 5:50 AM PST

Narrative

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for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/01/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.2	3.4 - 11.0 K/mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
RBC	2.45	3.98 - 5.25 M/mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
HGB	7.5	11.9 - 15.3 g/dL		02/01/2024 6:49 AM PST	MERCY LAB CV	
HCT	24.6	37.3 - 46.7 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/01/2024 6:49 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/01/2024 6:49 AM PST	MERCY LAB CV	
MCHC	30.5	31.0 - 36.0 g/dL		02/01/2024 6:49 AM PST	MERCY LAB CV	
RDW	13.5	11.7 - 14.9 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Platelet Count	573	150 - 425 K/mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		02/01/2024 6:49 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/01/2024 6:49 AM PST	MERCY LAB CV	
Neutrophils %	44.4	38.0 - 74.0 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
IG%	1.0	0.0 - 1.2 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Lymphocytes %	39.6	16.0 - 48.0 %		02/01/2024 6:49 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	9.8	4.9 - 12.5 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Eosinophils %	4.2	0.0 - 9.5 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.33	1.50 - 7.40 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
IG#	0.05	0.00 - 0.10 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.07	0.90 - 3.10 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Monocytes	0.51	0.26 - 0.87 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.22	0.00 - 0.51 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.33	1.50 - 7.40 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/01/2024 5:45 AM PST	02/01/2024 6:32 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/01/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/01/2024 7:09 AM PST	MERCY LAB CV	
Glucose	90	70 - 125 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Sodium	135	137 - 145 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	102	98 - 107 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Urea Nitrogen	17	7 - 17 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		02/01/2024 7:09 AM PST	MERCY LAB CV	
Anion Gap	-3	6 - 14 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Osmolality Calculated	281	280 - 305 mOsm/kg H20		02/01/2024 7:09 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Protein, total	6.8	6.3 - 8.2 g/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Alkaline Phosphatase	71	38 - 126 Units/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
AST	28	14 - 36 Units/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Bilirubin, Total	<0.1	0.2 - 1.3 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/01/2024 5:45 AM PST	02/01/2024 6:32 AM PST

Narrative

MERCY LAB CV - 02/01/2024 7:09 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/31/2024 6:10 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/31/2024 7:09 AM PST	MERCY LAB CV	
Glucose	92	70 - 125 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Sodium	135	137 - 145 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/31/2024 7:09 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Osmolality Calculated	282	280 - 305 mOsm/kg H20		01/31/2024 7:09 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Protein, total	7.3	6.3 - 8.2 g/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Albumin	3.0	3.5 - 5.0 g/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Alkaline Phosphatase	88	38 - 126 Units/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
AST	32	14 - 36 Units/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
ALT	17	<=34 Units/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:10 AM PST	01/31/2024 6:38 AM PST

Narrative

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MERCY LAB CV - 01/31/2024 7:09 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.6	3.4 - 11.0 K/mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
RBC	2.57	3.98 - 5.25 M/mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
HGB	8.0	11.9 - 15.3 g/dL		01/31/2024 7:35 AM PST	MERCY LAB CV	
HCT	26.1	37.3 - 46.7 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/31/2024 7:35 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/31/2024 7:35 AM PST	MERCY LAB CV	
MCHC	30.7	31.0 - 36.0 g/dL		01/31/2024 7:35 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Platelet Count	598	150 - 425 K/mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		01/31/2024 7:35 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/31/2024 7:35 AM PST	MERCY LAB CV	
Neutrophils %	45.8	38.0 - 74.0 %		01/31/2024 7:35 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
IG%	0.5	0.0 - 1.2 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Lymphocytes %	39.7	16.0 - 48.0 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Monocytes %	7.9	4.9 - 12.5 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Eosinophils %	5.0	0.0 - 9.5 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Basophils %	1.1	0.0 - 1.6 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.56	1.50 - 7.40 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.22	0.90 - 3.10 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Monocytes	0.44	0.26 - 0.87 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.28	0.00 - 0.51 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Basophils	0.06	0.00 - 0.09 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.56	1.50 - 7.40 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:38 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

IRON +TIBC (IRON AND TIBC) - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Iron	34	50 - 170 mcg/dL		01/31/2024 12:29 PM PST	SCRIPPS LAB	
Transferrin	128	180 - 382 mg/ dL		01/31/2024 12:29 PM PST	SCRIPPS LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Iron Binding Capacity Calculated	160	250 - 450 mcg/dL		01/31/2024 12:29 PM PST	SCRIPPS LAB	
Iron Saturation	21	20 - 50 %		01/31/2024 12:29 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:38 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

FOLATE - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Folate	10.5	7.0 - 31.4 ng/mL		01/31/2024 12:58 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:39 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

VITAMIN B12 - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Vitamin B-12	718	213 - 816 pg/mL		01/31/2024 12:58 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:39 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

IR PICC LINE PLACEMENT - US/FLUROR GUIDANCE AS NEEDED (IR PICC LINE PLACEMENT - US/FLUORO GUIDANCE AS NEEDED) - Final result (01/30/2024 4:05 PM PST)

Anatomical Region	Laterality	Modality
		IR/OR Hybrid Labs

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/30/2024 4:11 PM PST	

Impressions

01/30/2024 4:11 PM PST
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance.

release

Narrative

01/30/2024 4:11 PM PST
EXAMINATION: IR PICC LINE PLACEMENT - US/FLUORO GUIDANCE AS NEEDED

DATE/TIME: 1/30/2024 4:05 PM

HISTORY: for long-term antibiotic administration

COMPARISON: None.

TECHNIQUE: A description of the procedure, including risks, complications and benefits of the procedure were explained to the patient who gave written and oral consent. The skin around the access site was prepped using maximal sterile barrier preparation technique which included caps, masks, sterile gowns, and sterile gloves worn by all personnel. In addition, the following was also utilized: hand hygiene, a large sterile sheet/drape, and 2% chlorhexidine for cutaneous antisepsis. In addition, sterile gel and a sterile transducer probe cover was utilized. Real-time ultrasound was used to evaluate potential access sites for venous access, and was used to directly visualize needle entry into the selected vein lumen. Using local anesthetic, the access vein was punctured with a 21G needle and an 0.018 inch guidewire was placed. A 5F sheath was placed and the PICC line was advanced into position using fluoroscopic guidance. The line was secured to the skin and a sterile dressing was
applied.

FLUOROSCOPY: Time 0.1-minute, One fluoroscopic spot image(s) obtained

FINDINGS:
VEIN ACCESSED: Right basilic vein.
CATHETER: 4 French Power PICC
CATHETER TIP POSITION: Distal superior vena cava.
MEDICATIONS: Local anesthetic.
IMAGING FINDINGS: No sonographic or fluoroscopic abnormalities.
OTHER: None.

Procedure Note

Trevor Nelson, MD - 01/30/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: IR PICC LINE PLACEMENT - US/FLUORO GUIDANCE AS NEEDED

DATE/TIME: 1/30/2024 4:05 PM

HISTORY: for long-term antibiotic administration

COMPARISON: None.

TECHNIQUE: A description of the procedure, including risks, complications and benefits of the procedure were explained to the patient who gave written and oral consent. The skin around the access site was prepped using maximal sterile barrier preparation technique which included caps, masks, sterile gowns, and sterile gloves worn by all personnel. In addition, the following was also utilized: hand hygiene, a large sterile sheet/drape, and 2% chlorhexidine for cutaneous antisepsis. In addition, sterile gel and a sterile transducer probe cover was utilized. Real-time ultrasound was used to evaluate potential access sites for venous access, and was used to directly visualize needle entry into the selected vein lumen. Using local anesthetic, the access vein was punctured with a 21G needle and an 0.018 inch guidewire was placed. A 5F sheath was placed and the PICC line was advanced into position using fluoroscopic guidance. The line was secured to the skin and a sterile dressing was
applied.

FLUOROSCOPY: Time 0.1-minute, One fluoroscopic spot image(s) obtained

FINDINGS:
VEIN ACCESSED: Right basilic vein.
CATHETER: 4 French Power PICC
CATHETER TIP POSITION: Distal superior vena cava.
MEDICATIONS: Local anesthetic.
IMAGING FINDINGS: No sonographic or fluoroscopic abnormalities.
OTHER: None.

IMPRESSION:
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance.

release

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	IMG IR PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/30/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.3	3.4 - 11.0 K/mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
RBC	2.33	3.98 - 5.25 M/mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
HGB	7.1	11.9 - 15.3 g/dL		01/30/2024 5:50 AM PST	MERCY LAB CV	
HCT	23.8	37.3 - 46.7 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/30/2024 5:50 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/30/2024 5:50 AM PST	MERCY LAB CV	
MCHC	29.8	31.0 - 36.0 g/dL		01/30/2024 5:50 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
RDW	13.4	11.7 - 14.9 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Platelet Count	496	150 - 425 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		01/30/2024 5:50 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/30/2024 5:50 AM PST	MERCY LAB CV	
Neutrophils %	41.3	38.0 - 74.0 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
IG%	0.5	0.0 - 1.2 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Lymphocytes %	38.3	16.0 - 48.0 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Monocytes %	11.7	4.9 - 12.5 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Eosinophils %	7.0	0.0 - 9.5 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Basophils %	1.2	0.0 - 1.6 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Neutrophils	1.77	1.50 - 7.40 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.64	0.90 - 3.10 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Monocytes	0.50	0.26 - 0.87 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.30	0.00 - 0.51 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Preliminary Abs Neut	1.77	1.50 - 7.40 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/30/2024 5:21 AM PST	01/30/2024 5:43 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/30/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/30/2024 6:00 AM PST	MERCY LAB CV	
Glucose	100	70 - 125 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Sodium	135	137 - 145 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Urea Nitrogen	27	7 - 17 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Creatinine	1.22	0.50 - 1.00 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	51	>60 mL/min/1.73m*2		01/30/2024 6:00 AM PST	MERCY LAB CV	
Anion Gap	-1	6 - 14 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Osmolality Calculated	285	280 - 305 mOsm/kg H20		01/30/2024 6:00 AM PST	MERCY LAB CV	
Calcium	8.1	8.4 - 10.2 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Protein, total	6.4	6.3 - 8.2 g/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Alkaline Phosphatase	75	38 - 126 Units/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
AST	23	14 - 36 Units/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
ALT	14	<=34 Units/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/30/2024 5:21 AM PST	01/30/2024 5:40 AM PST

Narrative

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MERCY LAB CV - 01/30/2024 6:00 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (01/29/2024 4:18 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	>140	<=30 mm/hr		01/29/2024 5:42 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 4:18 PM PST	01/29/2024 4:38 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (01/29/2024 4:18 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	20.6	<10.0 mg/L		01/29/2024 4:58 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 4:18 PM PST	01/29/2024 4:38 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CT cervical spine with contrast (CT CERVICAL SPINE WITH CONTRAST) - Final result (01/29/2024 3:23 PM PST)

Anatomical Region	Laterality	Modality
C-spine		Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/29/2024 3:27 PM PST	

Impressions

01/29/2024 3:35 PM PST
IMPRESSION:

1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis.
2. Persistent prevertebral fluid, probably similar in extent to the recent MRI.

release

Narrative

01/29/2024 3:35 PM PST
EXAMINATION: CT CERVICAL SPINE WITH CONTRAST

DATE/TIME: 1/29/2024 3:23 PM

HISTORY: Prevertebral, retropharyngeal fluid collection

COMPARISON: MRI cervical spine 1/26/2024

TECHNIQUE: CT examination was performed with intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

There is reversal of cervical lordosis. There is C2-3 and C7-T1 mild anterolisthesis.

There are erosive changes of the C4-5 endplates. There is also irregularity of the C5-6 endplates.

There is persistent prevertebral fluid, extending from C2-C7.

There are multilevel degenerative changes, most pronounced at C6-7 where a disc osteophyte complex and uncovertebral hypertrophy cause mild canal stenosis and moderate left and mild right foraminal stenosis.

There is a partially visualized right pleural effusion.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 19; DLP: 317

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

Procedure Note

Benjamin Freeze, MD - 01/29/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: CT CERVICAL SPINE WITH CONTRAST

DATE/TIME: 1/29/2024 3:23 PM

HISTORY: Prevertebral, retropharyngeal fluid collection

COMPARISON: MRI cervical spine 1/26/2024

TECHNIQUE: CT examination was performed with intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

There is reversal of cervical lordosis. There is C2-3 and C7-T1 mild anterolisthesis.

There are erosive changes of the C4-5 endplates. There is also irregularity of the C5-6 endplates.

There is persistent prevertebral fluid, extending from C2-C7.

There are multilevel degenerative changes, most pronounced at C6-7 where a disc osteophyte complex and uncovertebral hypertrophy cause mild canal stenosis and moderate left and mild right foraminal stenosis.

There is a partially visualized right pleural effusion.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 19; DLP: 317

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

IMPRESSION:
IMPRESSION:

1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis.
2. Persistent prevertebral fluid, probably similar in extent to the recent MRI.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG CT PROCEDURES	Final Result

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (01/29/2024 2:59 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/29/2024 3:19 PM PST	

Impressions

01/29/2024 3:20 PM PST

Impressions

IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Narrative

01/29/2024 3:20 PM PST

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/29/2024 2:59 PM

HISTORY: bilateral leg edema

COMPARISON: 1/16/2024

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

Procedure Note

Paul Rickards, MD - 01/29/2024

Formatting of this note might be different from the original.

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/29/2024 2:59 PM

HISTORY: bilateral leg edema

COMPARISON: 1/16/2024

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

IMPRESSION:
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG US PROCEDURES	Final Result

X-ray chest 2 views with pa/ap and lateral (X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL) - Final result (01/29/2024 9:22 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/29/2024 9:26 AM PST	

Impressions

01/29/2024 9:26 AM PST
IMPRESSION: No significant change from prior.

release

Narrative

01/29/2024 9:26 AM PST
EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 1/29/2024 9:22 AM

HISTORY: f/u pleural fluid collection

COMPARISON: 1/25/2024

FINDINGS:

1. Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.
2. Clear left lung.
3. Stable cardiac silhouette size.

Procedure Note

Harold Pimenta, MD - 01/29/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 1/29/2024 9:22 AM

HISTORY: f/u pleural fluid collection

COMPARISON: 1/25/2024

FINDINGS:

1. Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.
2. Clear left lung.
3. Stable cardiac silhouette size.

IMPRESSION:
IMPRESSION: No significant change from prior.

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.6	3.4 - 11.0 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
RBC	2.48	3.98 - 5.25 M/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HGB	7.7	11.9 - 15.3 g/dL		01/29/2024 7:23 AM PST	MERCY LAB CV	
HCT	25.2	37.3 - 46.7 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/29/2024 7:23 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/29/2024 7:23 AM PST	MERCY LAB CV	
MCHC	30.6	31.0 - 36.0 g/dL		01/29/2024 7:23 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Platelet Count	552	150 - 425 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
MPV	8.4	9.0 - 12.8 fL		01/29/2024 7:23 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/29/2024 7:23 AM PST	MERCY LAB CV	
Neutrophils %	53.3	38.0 - 74.0 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
IG%	0.6	0.0 - 1.2 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Lymphocytes %	27.5	16.0 - 48.0 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Monocytes %	10.2	4.9 - 12.5 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Eosinophils %	7.1	0.0 - 9.5 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Basophils %	1.3	0.0 - 1.6 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.46	1.50 - 7.40 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.27	0.90 - 3.10 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Monocytes	0.47	0.26 - 0.87 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.33	0.00 - 0.51 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Basophils	0.06	0.00 - 0.09 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.46	1.50 - 7.40 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 7:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	>140	<=30 mm/hr		01/29/2024 11:20 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 7:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	26.8	<10.0 mg/L		01/29/2024 10:22 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 6:07 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/29/2024 6:52 AM PST	MERCY LAB CV	
Glucose	101	70 - 125 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Chloride	97	98 - 107 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Creatinine	0.75	0.50 - 1.00 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	92	>60 mL/min/1.73m*2		01/29/2024 6:52 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Osmolality Calculated	275	280 - 305 mOsm/kg H20		01/29/2024 6:52 AM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Protein, total	7.1	6.3 - 8.2 g/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Alkaline Phosphatase	86	38 - 126 Units/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
AST	25	14 - 36 Units/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/29/2024 6:52 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 6:07 AM PST

Narrative

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MERCY LAB CV - 01/29/2024 6:52 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

HIV ANTIGEN/ANTIBODY COMBO - Final result (01/28/2024 8:28 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HIV Antigen / Antibody Combo	Non Reactive	Non Reactive		01/28/2024 10:45 PM PST	SCRIPPS LAB	

Comment: HIV-1 p24 antigen and HIV-1/HIV-2 antibodies are not detected.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 8:28 PM PST	01/28/2024 8:31 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

CYCLIC CITRUL PEPTIDE AB, IGG (CYCLIC CITRUL PEPTIDE ANTIBODY, IGG) - Final result (01/28/2024 12:19 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cyclic Citrul Peptide Antibodies, IgG	1.0	<5.0 U/mL		01/28/2024 6:25 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 12:19 PM PST	01/28/2024 12:58 PM PST

Narrative

SCRIPPS LAB - 01/28/2024 6:25 PM PST
Alinity i Anti-CCP results should not be used interchangeably with other manufacturers' methods for anti-CCP determinations.

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

RHEUMATOID FACTOR, QUANTITATIVE - Final result (01/28/2024 12:19 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Rheumatoid Factor	25	<30 U/mL		01/28/2024 5:53 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 12:19 PM PST	01/28/2024 12:58 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

ANA (ANTI-NUCLEAR ANTIBODY) - Final result (01/28/2024 12:19 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ANA	Negative	Negative		01/29/2024 11:07 AM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 12:19 PM PST	01/28/2024 12:58 PM PST

Narrative

SCRIPPS LAB - 01/29/2024 11:07 AM PST
Performed using Bio-Rad Autoimmune qualitative enzyme immunoassay (EIA).

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

GLUCOSE, POC - Final result (01/28/2024 7:25 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	130	70 - 125 mg/dL		01/28/2024 7:27 AM PST	MERCY LAB CV	
Operator ID	126,288			01/28/2024 7:27 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/28/2024 7:25 AM PST	01/28/2024 7:27 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

IRON +TIBC (IRON AND TIBC) - Final result (01/28/2024 5:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Iron	37	50 - 170 mcg/dL		01/28/2024 5:58 PM PST	SCRIPPS LAB	
Transferrin	123	180 - 382 mg/dL		01/28/2024 5:58 PM PST	SCRIPPS LAB	
Iron Binding Capacity Calculated	154	250 - 450 mcg/dL		01/28/2024 5:58 PM PST	SCRIPPS LAB	
Iron Saturation	24	20 - 50 %		01/28/2024 5:58 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 5:40 AM PST	01/28/2024 6:10 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

FERRITIN - Final result (01/28/2024 5:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Ferritin	113.52	4.63 - 204.00 ng/mL		01/28/2024 6:14 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 5:40 AM PST	01/28/2024 6:10 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

COMPREHENSIVE METABOLIC PANEL - Final result (01/28/2024 5:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/28/2024 7:00 AM PST	MERCY LAB CV	
Glucose	97	70 - 125 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Sodium	130	137 - 145 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Chloride	94	98 - 107 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Creatinine	0.67	0.50 - 1.00 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	101	>60 mL/min/1.73m*2		01/28/2024 7:00 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Osmolality Calculated	270	280 - 305 mOsm/kg H20		01/28/2024 7:00 AM PST	MERCY LAB CV	
Calcium	8.4	8.4 - 10.2 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Protein, total	7.4	6.3 - 8.2 g/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Albumin	2.9	3.5 - 5.0 g/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Alkaline Phosphatase	81	38 - 126 Units/L		01/28/2024 7:00 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
AST	32	14 - 36 Units/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 5:40 AM PST	01/28/2024 6:10 AM PST

Narrative

MERCY LAB CV - 01/28/2024 7:00 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

HEMOGRAM - Final result (01/27/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.8	3.4 - 11.0 K/mcL		01/27/2024 7:03 AM PST	MERCY LAB CV	
RBC	2.42	3.98 - 5.25 M/mcL		01/27/2024 7:03 AM PST	MERCY LAB CV	
HGB	7.6	11.9 - 15.3 g/dL		01/27/2024 7:03 AM PST	MERCY LAB CV	
HCT	24.3	37.3 - 46.7 %		01/27/2024 7:03 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/27/2024 7:03 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/27/2024 7:03 AM PST	MERCY LAB CV	
MCHC	31.3	31.0 - 36.0 g/dL		01/27/2024 7:03 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/27/2024 7:03 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	452	150 - 425 K/mcL		01/27/2024 7:03 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/27/2024 5:53 AM PST	01/27/2024 6:50 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/27/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/27/2024 7:36 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Sodium	129	137 - 145 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Chloride	97	98 - 107 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Creatinine	0.63	0.50 - 1.00 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	102	>60 mL/min/1.73m*2		01/27/2024 7:36 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Osmolality Calculated	269	280 - 305 mOsm/kg H20		01/27/2024 7:36 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Protein, total	6.7	6.3 - 8.2 g/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Albumin	2.7	3.5 - 5.0 g/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Alkaline Phosphatase	71	38 - 126 Units/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
AST	26	14 - 36 Units/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/27/2024 5:53 AM PST	01/27/2024 6:48 AM PST

Narrative

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MERCY LAB CV - 01/27/2024 7:36 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/26/2024 12:06 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	131	70 - 125 mg/dL		01/26/2024 12:09 PM PST	MERCY LAB CV	
Operator ID	255,308			01/26/2024 12:09 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/26/2024 12:06 PM PST	01/26/2024 12:09 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/26/2024 10:32 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/31/2024 12:09 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/26/2024 10:32 AM PST	01/26/2024 11:13 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/26/2024 10:23 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/31/2024 11:08 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of part of right upper limb / Unknown	Venipuncture / Unknown	01/26/2024 10:23 AM PST	01/26/2024 10:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MRI cervical spine with and without contrast (MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST) - Final result (01/26/2024 10:08 AM PST)

Anatomical Region	Laterality	Modality
C-spine		Magnetic Resonance

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/26/2024 10:14 AM PST	

Impressions

01/26/2024 10:23 AM PST

Impressions

Impression:

Contrast imaging is very limited due to extensive motion.

Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema.

Findings are concerning for discitis/osteomyelitis at C4-C5.

release

Narrative

01/26/2024 10:23 AM PST

EXAMINATION: MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST

DATE/TIME: 1/26/2024 10:08 AM

HISTORY: concern for osteomyelitis from previous MRI without contrast

COMPARISON: Noncontrast MRI of the cervical spine performed 1 day prior.

TECHNIQUE: Multiplanar multisequence MRI examination was performed without and with contrast.

FINDINGS:

The exam is very limited due to extensive motion artifact.

Pre and postcontrast axial and postcontrast sagittal images were obtained.

There is a large amount of prevertebral edema with generalized enhancement.
This extends into the chest, anterior mediastinum, series 6 images 10 through 13. No discrete fluid collection.

There is also enhancement of the C4, C5 and C6 vertebral bodies with enhancement of the C4-C5 disc. The other discs are difficult to visualize due to severe disc height loss at C3-4, C5-6 and C6-7.

No definite abnormal enhancement within the spinal canal, epidural compartment..

Procedure Note

Anila Jonnala, MD - 01/26/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST

DATE/TIME: 1/26/2024 10:08 AM

HISTORY: concern for osteomyelitis from previous MRI without contrast

COMPARISON: Noncontrast MRI of the cervical spine performed 1 day prior.

TECHNIQUE: Multiplanar multisequence MRI examination was performed without and with contrast.

FINDINGS:

The exam is very limited due to extensive motion artifact.

Pre and postcontrast axial and postcontrast sagittal images were obtained.

There is a large amount of prevertebral edema with generalized enhancement.
This extends into the chest, anterior mediastinum, series 6 images 10 through 13. No discrete fluid collection.

There is also enhancement of the C4, C5 and C6 vertebral bodies with enhancement of the C4-C5 disc. The other discs are difficult to visualize due to severe disc height loss at C3-4, C5-6 and C6-7.

No definite abnormal enhancement within the spinal canal, epidural compartment..

IMPRESSION:
Impression:

Contrast imaging is very limited due to extensive motion.

Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema.

Findings are concerning for discitis/osteomyelitis at C4-C5.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG MRI PROCEDURES	Final Result

HEMOGRAM - Final result (01/26/2024 5:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.7	3.4 - 11.0 K/mcL		01/26/2024 7:56 AM PST	MERCY LAB CV	
RBC	2.67	3.98 - 5.25 M/mcL		01/26/2024 7:56 AM PST	MERCY LAB CV	
HGB	8.2	11.9 - 15.3 g/dL		01/26/2024 7:56 AM PST	MERCY LAB CV	
HCT	26.7	37.3 - 46.7 %		01/26/2024 7:56 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/26/2024 7:56 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/26/2024 7:56 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCHC	30.7	31.0 - 36.0 g/dL		01/26/2024 7:56 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/26/2024 7:56 AM PST	MERCY LAB CV	
Platelet Count	478	150 - 425 K/mcL		01/26/2024 7:56 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/26/2024 5:36 AM PST	01/26/2024 7:19 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/26/2024 5:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/26/2024 7:40 AM PST	MERCY LAB CV	
Glucose	100	70 - 125 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Chloride	98	98 - 107 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Creatinine	0.67	0.50 - 1.00 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	101	>60 mL/min/1.73m*2		01/26/2024 7:40 AM PST	MERCY LAB CV	
Anion Gap	1	6 - 14 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Osmolality Calculated	273	280 - 305 mOsm/kg H20		01/26/2024 7:40 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Calcium	8.4	8.4 - 10.2 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Protein, total	6.9	6.3 - 8.2 g/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Alkaline Phosphatase	89	38 - 126 Units/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
AST	26	14 - 36 Units/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
ALT	14	<=34 Units/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/26/2024 5:36 AM PST	01/26/2024 7:20 AM PST

Narrative

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MERCY LAB CV - 01/26/2024 7:40 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (01/25/2024 10:52 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	131	<=30 mm/hr		01/26/2024 12:38 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 10:52 PM PST	01/25/2024 10:54 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (01/25/2024 10:52 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	32.7	<10.0 mg/L		01/25/2024 11:11 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 10:52 PM PST	01/25/2024 10:54 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MRI cervical spine without contrast (MRI CERVICAL SPINE WITHOUT CONTRAST) - Final result (01/25/2024 6:33 PM PST)

Anatomical Region	Laterality	Modality
C-spine		Magnetic Resonance

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/25/2024 6:38 PM PST	

Impressions

01/25/2024 9:48 PM PST
IMPRESSION:
1. Abnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation.
2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.

Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024.

release 1419

A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

Narrative

01/25/2024 9:48 PM PST

Narrative

EXAMINATION: MRI CERVICAL SPINE WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:33 PM

HISTORY: Evaluate severe ongoing debilitating neck pain. Cough, fever and neck pain.

COMPARISON: None.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI of the cervical spine was performed without contrast.

FINDINGS:

Abnormal edema at the vertebral bodies of C4, C5, C6 with indistinct anterior cortex at C4, C6.
Abnormal prevertebral collection, fluid from C2-C7. Fluid in the disc space of C4-C5.
No epidural collection.
Mild posterior disc bulge C4-T4 identified. There is moderate canal narrowing from disc bulge and short pedicles at C4-C7. Mild central canal narrowing C7-T1, T1-T2.
Moderate facet arthrosis identified. Axial images are motion degraded.

Severe right neural foramen narrowing C4-C5, moderate neural foramen narrowing bilaterally C5-C6, C6-C7.

Procedure Note

David Vu, MD - 01/25/2024

Formatting of this note might be different from the original.

EXAMINATION: MRI CERVICAL SPINE WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:33 PM

HISTORY: Evaluate severe ongoing debilitating neck pain. Cough, fever and neck pain.

COMPARISON: None.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI of the cervical spine was performed without contrast.

FINDINGS:

Abnormal edema at the vertebral bodies of C4, C5, C6 with indistinct anterior cortex at C4, C6.
Abnormal prevertebral collection, fluid from C2-C7. Fluid in the disc space of C4-C5.
No epidural collection.
Mild posterior disc bulge C4-T4 identified. There is moderate canal narrowing from disc bulge and short pedicles at C4-C7. Mild central canal narrowing C7-T1, T1-T2.
Moderate facet arthrosis identified. Axial images are motion degraded.

Severe right neural foramen narrowing C4-C5, moderate neural foramen narrowing bilaterally C5-C6, C6-C7.

IMPRESSION:
IMPRESSION:
1. Abnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation.
2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.

Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024.

release 1419

A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG MRI PROCEDURES	Final Result

MRI brain without contrast (MRI BRAIN WITHOUT CONTRAST) - Final result (01/25/2024 6:07 PM PST)

Anatomical Region	Laterality	Modality
Head		Magnetic Resonance

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/25/2024 6:10 PM PST	

Impressions

01/25/2024 9:48 PM PST

IMPRESSION:
1. No acute intracranial findings.
2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change.
3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process.

release

Narrative

01/25/2024 9:48 PM PST

EXAMINATION: MRI BRAIN WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:07 PM

HISTORY: Severe occipital and neck headaches. Shortness of breath, pleural effusion.

COMPARISON: CT chest same day.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI examination was performed without contrast.

FINDINGS:
There is no acute infarct.

There is no intracranial hemorrhage. Punctate susceptibility at the left caudate body is probably from mineralization. Minimal FLAIR high signal in the white matter is likely from microvascular change. This is age-appropriate. The flow voids are intact.

Small amount of fluid in the right mastoid air cells. Mild mucosal thickening of the ethmoid air cells. The soft tissues are unremarkable.

Low signal within the diploic space, upper cervical spine from marrow replacing process or anemia.
Moderate degenerative change in the upper cervical spine at C3-C4, C4-C5. Moderate facet arthrosis in the upper cervical spine.
Moderate central canal narrowing C4-C5.

Procedure Note

David Vu, MD - 01/25/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: MRI BRAIN WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:07 PM

HISTORY: Severe occipital and neck headaches. Shortness of breath, pleural effusion.

COMPARISON: CT chest same day.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI examination was performed without contrast.

FINDINGS:
There is no acute infarct.

There is no intracranial hemorrhage. Punctate susceptibility at the left caudate body is probably from mineralization. Minimal FLAIR high signal in the white matter is likely from microvascular change. This is age-appropriate. The flow voids are intact.

Small amount of fluid in the right mastoid air cells. Mild mucosal thickening of the ethmoid air cells. The soft tissues are unremarkable.

Low signal within the diploic space, upper cervical spine from marrow replacing process or anemia.
Moderate degenerative change in the upper cervical spine at C3-C4, C4-C5. Moderate facet arthrosis in the upper cervical spine. Moderate central canal narrowing C4-C5.

IMPRESSION:
IMPRESSION:
1. No acute intracranial findings.
2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change.
3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG MRI PROCEDURES	Final Result

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/25/2024 1:14 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/25/2024 1:31 PM PST	

Impressions

01/25/2024 1:31 PM PST
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated

release

Narrative

01/25/2024 1:31 PM PST

Narrative

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EXAMINATION:  X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME:  1/25/2024 1:14 PM

HISTORY:  status post Right thoracentesis

COMPARISON: 1/22/2024

FINDINGS:

LUNGS: Basilar right compressive change with volume loss
VASCULATURE:  Normal. Unremarkable pulmonary vasculature.
CARDIAC:  Normal. No cardiac silhouette abnormality.
MEDIASTINUM:  Normal. No visible mass or adenopathy.
PLEURA: Ongoing moderate to large right effusion. No pneumothorax
BONES:  Normal. No fracture or visible bony lesion.
OTHER:  Negative.
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Procedure Note

Trevor Nelson, MD - 01/25/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/25/2024 1:14 PM

HISTORY: status post Right thoracentesis

COMPARISON: 1/22/2024

FINDINGS:

LUNGS: Basilar right compressive change with volume loss
VASCULATURE: Normal. Unremarkable pulmonary vasculature.
CARDIAC: Normal. No cardiac silhouette abnormality.
MEDIASTINUM: Normal. No visible mass or adenopathy.
PLEURA: Ongoing moderate to large right effusion. No pneumothorax
BONES: Normal. No fracture or visible bony lesion.
OTHER: Negative.

IMPRESSION:
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

PH, PLEURAL FLUID - Final result (01/25/2024 10:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Pleural Fluid	7.27	Not Established		01/25/2024 10:51 AM PST	MERCY LAB CV	
Operator ID	202,545			01/25/2024 10:51 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:35 AM PST	01/25/2024 10:43 AM PST

Narrative

MERCY LAB CV - 01/25/2024 10:51 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PATHOLOGIST FLUID COMMENT - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Pathologist Review, CSF/ Body Fluids	Morphologic analysis of the smear shows severe acute inflammation with PMN vacuoles without atypia. Carla Stayboldt, M.D.			01/25/2024 4:18 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, WOUND (WOUND/FLUID/TISSUE CULTURE W/STAIN) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Wound Culture	No Growth Final		MIC	01/29/2024 7:46 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	Many WBCs			01/29/2024 7:46 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	No organisms seen			01/29/2024 7:46 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

BODY FLUID CELL COUNT WITH DIFF (BODY FLUID CELL COUNT WITH DIFFERENTIAL) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Volume (ML)	265.0	mL		01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Color	Orange			01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Appearance	Slightly Hazy			01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Nucleated Cell Count	6,055	/mcL		01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid RBC Count	17,000	/mcL		01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Neutrophils %	81	%		01/25/2024 2:42 PM PST	MERCY LAB CV	
Comment: Appended report. These results have been appended to a previously preliminary verified report.						
Fluid Lymphocytes %	10	%		01/25/2024 2:42 PM PST	MERCY LAB CV	
Comment: Appended report. These results have been appended to a previously preliminary verified report.						
Fluid Mononuclears %	9	%		01/25/2024 2:42 PM PST	MERCY LAB CV	
Comment: Appended report. These results have been appended to a previously preliminary verified report.						

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 2:42 PM PST
Reference Interval has not been established for this specimen type.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTEIN BODY FLUID (PROTEIN, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/25/2024 12:25 PM PST	MERCY LAB CV	
Protein (FL)	4.3	Not Established g/dL		01/25/2024 12:25 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

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MERCY LAB CV - 01/25/2024 12:25 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories.
The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for clinical purposes.
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Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTATE DEHYDROGENASE BODY FLUID (LACTATE DEHYDROGENASE, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/25/2024 1:52 PM PST	MERCY LAB CV	
Lactate Dehrdrogenase (FL)	1,862	Not Established Units/L		01/25/2024 1:52 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

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MERCY LAB CV - 01/25/2024 1:52 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories.
The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for clinical purposes.
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Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE BODY FLUID (GLUCOSE, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/25/2024 12:25 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose (FL)	65	Not Established mg/dL		01/25/2024 12:25 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 12:25 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

ALBUMIN BODY FLUID (ALBUMIN, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Albumin (FL)	1.7	Not Established g/dL		01/25/2024 12:25 PM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/25/2024 12:25 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 12:25 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

HEMOGRAM - Final result (01/25/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.6	3.4 - 11.0 K/mcL		01/25/2024 6:26 AM PST	MERCY LAB CV	
RBC	2.54	3.98 - 5.25 M/mcL		01/25/2024 6:26 AM PST	MERCY LAB CV	
HGB	7.8	11.9 - 15.3 g/dL		01/25/2024 6:26 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HCT	25.4	37.3 - 46.7 %		01/25/2024 6:26 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/25/2024 6:26 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/25/2024 6:26 AM PST	MERCY LAB CV	
MCHC	30.7	31.0 - 36.0 g/dL		01/25/2024 6:26 AM PST	MERCY LAB CV	
RDW	13.1	11.7 - 14.9 %		01/25/2024 6:26 AM PST	MERCY LAB CV	
Platelet Count	413	150 - 425 K/mcL		01/25/2024 6:26 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 5:31 AM PST	01/25/2024 6:01 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTATE DEHYDROGENASE (LACTATE DEHYDROGENASE, TOTAL) - Final result (01/25/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Lactate Dehydrogenase	141	120 - 246 Units/L		01/25/2024 6:27 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 5:31 AM PST	01/25/2024 5:59 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/25/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/25/2024 6:28 AM PST	MERCY LAB CV	
Glucose	120	70 - 125 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Chloride	97	98 - 107 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/25/2024 6:28 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Osmolality Calculated	276	280 - 305 mOsm/kg H20		01/25/2024 6:28 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Protein, total	6.7	6.3 - 8.2 g/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Alkaline Phosphatase	74	38 - 126 Units/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
AST	27	14 - 36 Units/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 5:31 AM PST	01/25/2024 5:59 AM PST

Narrative

MERCY LAB CV - 01/25/2024 6:28 AM PST

Narrative

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for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/24/2024 5:48 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.8	3.4 - 11.0 K/mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
RBC	2.58	3.98 - 5.25 M/mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
HGB	8.0	11.9 - 15.3 g/dL		01/24/2024 6:30 AM PST	MERCY LAB CV	
HCT	26.3	37.3 - 46.7 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/24/2024 6:30 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/24/2024 6:30 AM PST	MERCY LAB CV	
MCHC	30.4	31.0 - 36.0 g/dL		01/24/2024 6:30 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Platelet Count	364	150 - 425 K/mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
MPV	8.6	9.0 - 12.8 fL		01/24/2024 6:30 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/24/2024 6:30 AM PST	MERCY LAB CV	
Neutrophils %	65.7	38.0 - 74.0 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
IG%	0.4	0.0 - 1.2 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Lymphocytes %	19.3	16.0 - 48.0 %		01/24/2024 6:30 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	11.1	4.9 - 12.5 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Eosinophils %	2.7	0.0 - 9.5 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.13	1.50 - 7.40 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Lymphocytes	0.92	0.90 - 3.10 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Monocytes	0.53	0.26 - 0.87 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.13	0.00 - 0.51 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Preliminary Abs Neut	3.13	1.50 - 7.40 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/24/2024 5:48 AM PST	01/24/2024 6:21 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/24/2024 5:48 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/24/2024 6:58 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Potassium	4.9	3.5 - 5.1 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	98	98 - 107 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/24/2024 6:58 AM PST	MERCY LAB CV	
Anion Gap	-3	6 - 14 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Osmolality Calculated	275	280 - 305 mOsm/kg H20		01/24/2024 6:58 AM PST	MERCY LAB CV	
Calcium	8.0	8.4 - 10.2 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Protein, total	6.6	6.3 - 8.2 g/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Alkaline Phosphatase	74	38 - 126 Units/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
AST	25	14 - 36 Units/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
ALT	17	<=34 Units/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/24/2024 5:48 AM PST	01/24/2024 6:23 AM PST

Narrative

MERCY LAB CV - 01/24/2024 6:58 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/23/2024 5:44 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.2	3.4 - 11.0 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
RBC	2.52	3.98 - 5.25 M/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
HGB	7.8	11.9 - 15.3 g/dL		01/23/2024 6:58 AM PST	MERCY LAB CV	
HCT	25.6	37.3 - 46.7 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/23/2024 6:58 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/23/2024 6:58 AM PST	MERCY LAB CV	
MCHC	30.5	31.0 - 36.0 g/dL		01/23/2024 6:58 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Platelet Count	353	150 - 425 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
MPV	8.8	9.0 - 12.8 fL		01/23/2024 6:58 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/23/2024 6:58 AM PST	MERCY LAB CV	
Neutrophils %	57.0	38.0 - 74.0 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
IG%	0.7	0.0 - 1.2 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Lymphocytes %	25.2	16.0 - 48.0 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Monocytes %	14.2	4.9 - 12.5 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Eosinophils %	2.2	0.0 - 9.5 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.37	1.50 - 7.40 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	1.05	0.90 - 3.10 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Monocytes	0.59	0.26 - 0.87 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.09	0.00 - 0.51 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.37	1.50 - 7.40 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/23/2024 5:44 AM PST	01/23/2024 6:20 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/23/2024 5:44 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/23/2024 6:46 AM PST	MERCY LAB CV	
Glucose	92	70 - 125 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Sodium	128	137 - 145 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Potassium	4.1	3.5 - 5.1 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		01/23/2024 6:46 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Anion Gap	-5	6 - 14 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Osmolality Calculated	267	280 - 305 mOsm/kg H20		01/23/2024 6:46 AM PST	MERCY LAB CV	
Calcium	7.8	8.4 - 10.2 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Protein, total	6.3	6.3 - 8.2 g/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Albumin	2.5	3.5 - 5.0 g/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Alkaline Phosphatase	68	38 - 126 Units/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
AST	25	14 - 36 Units/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
ALT	14	<=34 Units/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/23/2024 5:44 AM PST	01/23/2024 6:20 AM PST

Narrative

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MERCY LAB CV - 01/23/2024 6:46 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/22/2024 12:09 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/22/2024 12:25 PM PST	

Impressions

01/22/2024 12:26 PM PST

IMPRESSION:
1. Right chest tube removal with no pneumothorax. There is stable pleural-parenchymal opacity at the right lung base.

2. Mild left basilar atelectatic versus low-grade inflammatory change.

release
CC Dr: p: f:

Narrative

01/22/2024 12:26 PM PST

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/22/2024 12:09 PM

HISTORY: dyspnea

COMPARISON: 1/21/2024

FINDINGS: Interval removal of the right chest tube. There is continued moderate pleural thickening at the right lung base with associated mild to moderate patchy parenchymal opacity and streak-like change. Minimal streak-like changes are seen at the left lung base with minimal blunting of left lateral costophrenic angle.

Stable mild cardiomegaly. No pneumothorax or pulmonary venous hypertension.

Procedure Note

Ross Christensen, MD - 01/22/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/22/2024 12:09 PM

HISTORY: dyspnea

COMPARISON: 1/21/2024

FINDINGS: Interval removal of the right chest tube. There is continued moderate pleural thickening at the right lung base with associated mild to moderate patchy parenchymal opacity and streak-like change. Minimal streak-like changes are seen at the left lung base with minimal blunting of left lateral costophrenic angle.

Stable mild cardiomegaly. No pneumothorax or pulmonary venous hypertension.

IMPRESSION:
IMPRESSION:
1. Right chest tube removal with no pneumothorax. There is stable pleural-parenchymal opacity at the right lung base.

2. Mild left basilar atelectatic versus low-grade inflammatory change.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Diep Dao DO	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/22/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.8	3.4 - 11.0 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
RBC	2.69	3.98 - 5.25 M/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
HGB	8.4	11.9 - 15.3 g/dL		01/22/2024 6:44 AM PST	MERCY LAB CV	
HCT	26.8	37.3 - 46.7 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/22/2024 6:44 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/22/2024 6:44 AM PST	MERCY LAB CV	
MCHC	31.3	31.0 - 36.0 g/dL		01/22/2024 6:44 AM PST	MERCY LAB CV	
RDW	13.3	11.7 - 14.9 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Platelet Count	368	150 - 425 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
MPV	8.9	9.0 - 12.8 fL		01/22/2024 6:44 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/22/2024 6:44 AM PST	MERCY LAB CV	
Neutrophils %	67.6	38.0 - 74.0 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
IG%	0.4	0.0 - 1.2 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Lymphocytes %	17.6	16.0 - 48.0 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Monocytes %	12.2	4.9 - 12.5 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Eosinophils %	1.2	0.0 - 9.5 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.27	1.50 - 7.40 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Lymphocytes	0.85	0.90 - 3.10 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Monocytes	0.59	0.26 - 0.87 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.06	0.00 - 0.51 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Preliminary Abs Neut	3.27	1.50 - 7.40 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/22/2024 5:31 AM PST	01/22/2024 5:56 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/22/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/22/2024 7:13 AM PST	MERCY LAB CV	
Glucose	105	70 - 125 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
CO2, Total	35	22 - 30 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Urea Nitrogen	15	7 - 17 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Creatinine	0.86	0.50 - 1.00 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	78	>60 mL/min/1.73m*2		01/22/2024 7:13 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality Calculated	273	280 - 305 mOsm/kg H20		01/22/2024 7:13 AM PST	MERCY LAB CV	
Calcium	7.9	8.4 - 10.2 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Protein, total	5.9	6.3 - 8.2 g/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Albumin	2.4	3.5 - 5.0 g/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Alkaline Phosphatase	61	38 - 126 Units/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
AST	23	14 - 36 Units/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
ALT	11	<=34 Units/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/22/2024 5:31 AM PST	01/22/2024 5:56 AM PST

Narrative

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MERCY LAB CV - 01/22/2024 7:13 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SODIUM - Final result (01/21/2024 3:51 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	126	137 - 145 mmol/L		01/21/2024 4:07 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 3:51 PM PST	01/21/2024 3:55 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

OSMOLALITY - Final result (01/21/2024 3:51 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality	281	275 - 295 mOsm/kg H2O		01/21/2024 7:57 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 3:51 PM PST	01/21/2024 3:55 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Jennifer Pham MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/21/2024 5:43 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/21/2024 9:32 AM PST	

Impressions

01/21/2024 9:34 AM PST
IMPRESSION:

Right pleural catheter in place.

Small to moderate pleural effusion and patchy right basilar opacity, probably atelectasis.

No pneumothorax.

release

Narrative

01/21/2024 9:34 AM PST

Narrative

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EXAMINATION:  X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME:  1/21/2024 5:43 AM

HISTORY:  Respiratory failure, not elsewhere classified

COMPARISON: 1/20/2024.

FINDINGS:

There is a right pleural catheter in place. A small to moderate right pleural effusion is not
significantly changed. There is no pneumothorax. Patchy right basilar opacity is noted.

The left lung field is clear. There is a trace left pleural effusion.

The cardiac silhouette is magnified.
```

Procedure Note

Tomer Roth, MD - 01/21/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/21/2024 5:43 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/20/2024.

FINDINGS:

There is a right pleural catheter in place. A small to moderate right pleural effusion is not significantly changed. There is no pneumothorax. Patchy right basilar opacity is noted.

The left lung field is clear. There is a trace left pleural effusion.

The cardiac silhouette is magnified.

IMPRESSION:
IMPRESSION:

Right pleural catheter in place.

Small to moderate pleural effusion and patchy right basilar opacity, probably atelectasis.

No pneumothorax.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.7	3.4 - 11.0 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
RBC	2.74	3.98 - 5.25 M/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
HGB	8.6	11.9 - 15.3 g/dL		01/21/2024 6:00 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HCT	27.4	37.3 - 46.7 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/21/2024 6:00 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/21/2024 6:00 AM PST	MERCY LAB CV	
MCHC	31.4	31.0 - 36.0 g/dL		01/21/2024 6:00 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Platelet Count	404	150 - 425 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
MPV	8.9	9.0 - 12.8 fL		01/21/2024 6:00 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/21/2024 6:00 AM PST	MERCY LAB CV	
Neutrophils %	67.4	38.0 - 74.0 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
IG%	0.7	0.0 - 1.2 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Lymphocytes %	19.9	16.0 - 48.0 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Monocytes %	10.0	4.9 - 12.5 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Eosinophils %	1.1	0.0 - 9.5 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Basophils %	0.9	0.0 - 1.6 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.83	1.50 - 7.40 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.13	0.90 - 3.10 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Monocytes	0.57	0.26 - 0.87 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.06	0.00 - 0.51 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Preliminary Abs Neut	3.83	1.50 - 7.40 K/ mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CYCLIC CITRUL PEPTIDE AB, IGG (CYCLIC CITRUL PEPTIDE ANTIBODY, IGG) - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cyclic Citrul Peptide Antibodies, IgG	0.7	<5.0 U/ mL		01/21/2024 6:17 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:45 AM PST

Narrative

SCRIPPS LAB - 01/21/2024 6:17 PM PST
Alinity i Anti-CCP results should not be used interchangeably with other manufacturers' methods for anti-CCP determinations.

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

RHEUMATOID FACTOR, QUANTITATIVE - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Rheumatoid Factor	19	<30 U/ mL		01/21/2024 6:28 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:47 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

ANA (ANTI-NUCLEAR ANTIBODY) - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ANA	Negative	Negative		01/22/2024 11:18 AM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:47 AM PST

Narrative

SCRIPPS LAB - 01/22/2024 11:18 AM PST
Performed using Bio-Rad Autoimmune qualitative enzyme immunoassay (EIA).

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

COMPREHENSIVE METABOLIC PANEL - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/21/2024 6:08 AM PST	MERCY LAB CV	
Glucose	129	70 - 125 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Sodium	127	137 - 145 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Urea Nitrogen	10	7 - 17 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/21/2024 6:08 AM PST	MERCY LAB CV	
Anion Gap	-5	6 - 14 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Osmolality Calculated	265	280 - 305 mOsm/kg H20		01/21/2024 6:08 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Calcium	8.1	8.4 - 10.2 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Protein, total	6.0	6.3 - 8.2 g/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Albumin	2.4	3.5 - 5.0 g/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Alkaline Phosphatase	57	38 - 126 Units/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
AST	21	14 - 36 Units/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:45 AM PST

Narrative

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MERCY LAB CV - 01/21/2024 6:08 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

OSMOLALITY,URINE (OSMOLALITY, URINE) - Final result (01/20/2024 3:47 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality (UR)	152	50 - 1,200 mOsm/kg H2O		01/20/2024 4:43 PM PST	MERCY LAB CV	

Comment: Note: New Reference Range

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/20/2024 3:47 PM PST	01/20/2024 4:24 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SODIUM, RANDOM URINE - Final result (01/20/2024 3:46 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium, (Ur)	17	Not Established mmol/L		01/20/2024 4:38 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Voided urine specimen / Unknown		01/20/2024 3:46 PM PST	01/20/2024 4:24 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CREATININE, RANDOM URINE - Final result (01/20/2024 3:46 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Creatinine, (UR)	15.62	Not Established mg/dL		01/20/2024 4:38 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Voided urine specimen / Unknown		01/20/2024 3:46 PM PST	01/20/2024 4:24 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/20/2024 6:48 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/20/2024 8:06 AM PST	

Impressions

01/20/2024 8:07 AM PST

Impressions

IMPRESSION: Right-sided pigtail chest tube remains in place. There is minimal basilar volume loss. Cardiomegaly again noted. Mild redistribution.

release
CC Dr: p: f:

Narrative

01/20/2024 8:07 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 6:48 AM

HISTORY: Respiratory failure.

COMPARISON: 0112 hours.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

Procedure Note

Richard Abello, MD - 01/20/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 6:48 AM

HISTORY: Respiratory failure.

COMPARISON: 0112 hours.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

IMPRESSION:
IMPRESSION: Right-sided pigtail chest tube remains in place. There is minimal basilar volume loss. Cardiomegaly again noted. Mild redistribution.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/20/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	7.6	3.4 - 11.0 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
RBC	3.12	3.98 - 5.25 M/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
HGB	9.8	11.9 - 15.3 g/dL		01/20/2024 6:07 AM PST	MERCY LAB CV	
HCT	31.6	37.3 - 46.7 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		01/20/2024 6:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCH	31	26 - 33 pg		01/20/2024 6:07 AM PST	MERCY LAB CV	
MCHC	31.0	31.0 - 36.0 g/dL		01/20/2024 6:07 AM PST	MERCY LAB CV	
RDW	13.6	11.7 - 14.9 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Platelet Count	466	150 - 425 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
MPV	9.0	9.0 - 12.8 fL		01/20/2024 6:07 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/20/2024 6:07 AM PST	MERCY LAB CV	
Neutrophils %	73.8	38.0 - 74.0 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Lymphocytes %	15.5	16.0 - 48.0 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Monocytes %	8.7	4.9 - 12.5 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Eosinophils %	0.5	0.0 - 9.5 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	5.61	1.50 - 7.40 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
IG#	0.06	0.00 - 0.10 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.18	0.90 - 3.10 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.66	0.26 - 0.87 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.04	0.00 - 0.51 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	5.61	1.50 - 7.40 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/20/2024 5:35 AM PST	01/20/2024 6:01 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/20/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/20/2024 6:42 AM PST	MERCY LAB CV	
Glucose	96	70 - 125 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Sodium	129	137 - 145 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Potassium	4.9	3.5 - 5.1 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
CO2, Total	35	22 - 30 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Urea Nitrogen	12	7 - 17 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Creatinine	0.66	0.50 - 1.00 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	101	>60 mL/min/1.73m*2		01/20/2024 6:42 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Osmolality Calculated	268	280 - 305 mOsm/kg H20		01/20/2024 6:42 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Protein, total	6.2	6.3 - 8.2 g/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Albumin	2.5	3.5 - 5.0 g/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Alkaline Phosphatase	64	38 - 126 Units/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
AST	18	14 - 36 Units/L		01/20/2024 6:42 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ALT	12	<=34 Units/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/20/2024 5:35 AM PST	01/20/2024 6:01 AM PST

Narrative

MERCY LAB CV - 01/20/2024 6:42 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/20/2024 1:22 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/20/2024 8:50 AM PST	

Impressions

01/20/2024 8:51 AM PST
IMPRESSION: Stable position of right chest tube, some loculated pleural fluid and no pneumothorax. Mild bibasilar volume loss.

These findings are concordant with the preliminary interpretation provided by StatRad radiologist,

Statrad Code: A

release
CC Dr: p: f:

Narrative

01/20/2024 8:51 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 1:22 AM

HISTORY: Pain/cough/SOB/infiltrate.

COMPARISON: Prior day.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

Procedure Note

Richard Abello, MD - 01/20/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 1:22 AM

HISTORY: Pain/cough/SOB/infiltrate.

COMPARISON: Prior day.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

IMPRESSION:
IMPRESSION: Stable position of right chest tube, some loculated pleural fluid and no pneumothorax. Mild bibasilar volume loss.

These findings are concordant with the preliminary interpretation provided by StatRad radiologist,

Statrad Code: A

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Jose Pena MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

X-ray foot right 3 or more views (X-RAY FOOT RIGHT 3 OR MORE VIEWS) - Final result (01/19/2024 3:51 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities, Foot	Right	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/19/2024 4:19 PM PST	

Impressions

01/19/2024 4:20 PM PST
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

release

Narrative

01/19/2024 4:20 PM PST
EXAMINATION: X-RAY FOOT RIGHT 3 OR MORE VIEWS

DATE/TIME: 1/19/2024 3:51 PM

HISTORY: pain and swelling.

COMPARISON: None.

FINDINGS:

Chronic erosions involve the distal first metatarsal with sclerotic margins. No acute pathology. No evidence of fracture. No lytic changes.

Hallux valgus.

Procedure Note

Brian Shore, MD - 01/19/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY FOOT RIGHT 3 OR MORE VIEWS

DATE/TIME: 1/19/2024 3:51 PM

HISTORY: pain and swelling.

COMPARISON: None.

FINDINGS:

Chronic erosions involve the distal first metatarsal with sclerotic margins. No acute pathology. No evidence of fracture. No lytic changes.

Hallux valgus.

IMPRESSION:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

release

Authorizing Provider	Result Type	Result Status
Diep Dao DO	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/19/2024 2:25 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/19/2024 2:45 PM PST	

Impressions

01/19/2024 2:46 PM PST
IMPRESSION: No change.

release

Narrative

01/19/2024 2:46 PM PST
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 2:25 PM

HISTORY: Respiratory failure

COMPARISON: 1/19/2024, earlier time.

FINDINGS:

1. Stable position of smallbore right chest tube. No discernible pneumothorax.
2. Persistent small peripheral right pleural effusion.
3. Unchanged haziness in the right lung base.
4. The left lung is clear. Stable cardiomediastinal silhouette.

Procedure Note

Harold Pimenta, MD - 01/19/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 2:25 PM

HISTORY: Respiratory failure

COMPARISON: 1/19/2024, earlier time.

FINDINGS:

1. Stable position of smallbore right chest tube. No discernible pneumothorax.
2. Persistent small peripheral right pleural effusion.
3. Unchanged haziness in the right lung base.
4. The left lung is clear. Stable cardiomediastinal silhouette.

IMPRESSION:
IMPRESSION: No change.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

SURVEILLANCE MRSA CULTURE - Final result (01/19/2024 12:30 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Staph Screen culture	NO Methicillin/ Oxacillin Resistant Staphylococcus aureus		MIC	01/20/2024 2:12 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasal structure / Unknown		01/19/2024 12:30 PM PST	01/19/2024 12:55 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/19/2024 6:38 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/19/2024 7:51 AM PST	

Impressions

01/19/2024 8:17 AM PST

IMPRESSION: No significant change in a right-sided chest tube, a moderate right pleural effusion, right lower lung field infiltrates.

release

Narrative

01/19/2024 8:17 AM PST

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 6:38 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: Chest x-ray 1/18/2024, CT chest 1/16/2024

FINDINGS:
Right-sided chest tube remains with its tip unchanged within the peripheral aspect of the right midlung field.
No significant change in the extent of a moderate right pleural effusion, patchy infiltrate within the right middle lobe and right lower lobe.

The left lung is clear.

Stable cardiomegaly and mediastinal contour.

Procedure Note

Anila Jonnala, MD - 01/19/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 6:38 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: Chest x-ray 1/18/2024, CT chest 1/16/2024

FINDINGS:
Right-sided chest tube remains with its tip unchanged within the peripheral aspect of the right midlung field.
No significant change in the extent of a moderate right pleural effusion, patchy infiltrate within the right middle lobe and right lower lobe.

The left lung is clear.

Stable cardiomegaly and mediastinal contour.

IMPRESSION:
IMPRESSION: No significant change in a right-sided chest tube, a moderate right pleural effusion, right lower lung field infiltrates.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/19/2024 4:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	9.6	3.4 - 11.0 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
RBC	3.21	3.98 - 5.25 M/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
HGB	10.2	11.9 - 15.3 g/dL		01/19/2024 7:09 AM PST	MERCY LAB CV	
HCT	32.8	37.3 - 46.7 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/19/2024 7:09 AM PST	MERCY LAB CV	
MCH	32	26 - 33 pg		01/19/2024 7:09 AM PST	MERCY LAB CV	
MCHC	31.1	31.0 - 36.0 g/dL		01/19/2024 7:09 AM PST	MERCY LAB CV	
RDW	13.7	11.7 - 14.9 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Platelet Count	426	150 - 425 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
MPV				01/19/2024 7:09 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/19/2024 7:09 AM PST	MERCY LAB CV	
Neutrophils %	78.9	38.0 - 74.0 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
IG%	0.9	0.0 - 1.2 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Lymphocytes %	11.4	16.0 - 48.0 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Monocytes %	7.6	4.9 - 12.5 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Eosinophils %	0.7	0.0 - 9.5 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Basophils %	0.5	0.0 - 1.6 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Neutrophils	7.56	1.50 - 7.40 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
IG#	0.09	0.00 - 0.10 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.09	0.90 - 3.10 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Monocytes	0.73	0.26 - 0.87 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.07	0.00 - 0.51 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Preliminary Abs Neut	7.56	1.50 - 7.40 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/19/2024 4:22 AM PST	01/19/2024 5:33 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URIC ACID - Final result (01/19/2024 4:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Uric Acid	2.6	2.5 - 6.2 mg/dL		01/19/2024 12:58 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/19/2024 4:22 AM PST	01/19/2024 5:33 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/19/2024 4:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/19/2024 6:14 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	130	137 - 145 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Potassium	4.7	3.5 - 5.1 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Creatinine	0.60	0.50 - 1.00 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	103	>60 mL/min/1.73m*2		01/19/2024 6:14 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Osmolality Calculated	270	280 - 305 mOsm/kg H20		01/19/2024 6:14 AM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Protein, total	6.2	6.3 - 8.2 g/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Alkaline Phosphatase	62	38 - 126 Units/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
AST	24	14 - 36 Units/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
ALT	13	<=34 Units/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/19/2024 4:22 AM PST	01/19/2024 5:33 AM PST

Narrative

MERCY LAB CV - 01/19/2024 6:14 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/18/2024 9:38 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/18/2024 9:39 AM PST	

Impressions

01/18/2024 9:40 AM PST
IMPRESSION: Stable pleural-parenchymal opacity the right lung base with no pneumothorax.

release
CC Dr: p: f:

Narrative

01/18/2024 9:40 AM PST
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/18/2024 9:38 AM

HISTORY: empyema

COMPARISON: 1/17/2024

FINDINGS: Stable position of the pigtail right chest tube overlying the periphery the right midlung zone. There is ongoing mild right lateral pleural thickening with blunting of the right lateral costophrenic angle, and continued mild hazy and streak-like opacity at the right lung base heart size is at the upper limits of normal with no new mediastinal or hilar abnormality. No pneumothorax.

Procedure Note

Ross Christensen, MD - 01/18/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/18/2024 9:38 AM

HISTORY: empyema

COMPARISON: 1/17/2024

FINDINGS: Stable position of the pigtail right chest tube overlying the periphery the right midlung zone. There is ongoing mild right lateral pleural thickening with blunting of the right lateral costophrenic angle, and continued mild hazy and streak-like opacity at the right lung base heart size is at the upper limits of normal with no new mediastinal or hilar abnormality. No pneumothorax.

IMPRESSION:
IMPRESSION: Stable pleural-parenchymal opacity the right lung base with no pneumothorax.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/18/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	11.6	3.4 - 11.0 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
RBC	3.29	3.98 - 5.25 M/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
HGB	10.1	11.9 - 15.3 g/dL		01/18/2024 7:25 AM PST	MERCY LAB CV	
HCT	34.2	37.3 - 46.7 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
MCV	104	81 - 100 fL		01/18/2024 7:25 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/18/2024 7:25 AM PST	MERCY LAB CV	
MCHC	29.5	31.0 - 36.0 g/dL		01/18/2024 7:25 AM PST	MERCY LAB CV	
RDW	14.0	11.7 - 14.9 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Platelet Count	450	150 - 425 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
MPV	9.5	9.0 - 12.8 fL		01/18/2024 7:25 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
nRBC	0.0	0.0 /100 WBCs		01/18/2024 7:25 AM PST	MERCY LAB CV	
Neutrophils %	80.2	38.0 - 74.0 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
IG%	1.0	0.0 - 1.2 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Lymphocytes %	11.8	16.0 - 48.0 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Monocytes %	6.3	4.9 - 12.5 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Eosinophils %	0.3	0.0 - 9.5 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Basophils %	0.4	0.0 - 1.6 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Neutrophils	9.27	1.50 - 7.40 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
IG#	0.12	0.00 - 0.10 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.36	0.90 - 3.10 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Monocytes	0.73	0.26 - 0.87 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.04	0.00 - 0.51 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Preliminary Abs Neut	9.27	1.50 - 7.40 K/ mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/18/2024 5:19 AM PST	01/18/2024 5:38 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/18/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/18/2024 6:06 AM PST	MERCY LAB CV	
Glucose	91	70 - 125 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Creatinine	0.60	0.50 - 1.00 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	103	>60 mL/min/1.73m*2		01/18/2024 6:06 AM PST	MERCY LAB CV	
Anion Gap	1	6 - 14 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Osmolality Calculated	274	280 - 305 mOsm/kg H20		01/18/2024 6:06 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Protein, total	6.5	6.3 - 8.2 g/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Alkaline Phosphatase	70	38 - 126 Units/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
AST	24	14 - 36 Units/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/18/2024 5:19 AM PST	01/18/2024 5:38 AM PST

Narrative

MERCY LAB CV - 01/18/2024 6:06 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (01/17/2024 5:22 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	97	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	97	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	160	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	96	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	338	ms			SCRIPPS RADIOLOGY	
QTC	429	ms			SCRIPPS RADIOLOGY	
P AXIS	63	degrees			SCRIPPS RADIOLOGY	
R AXIS	44	degrees			SCRIPPS RADIOLOGY	
T AXIS	101	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/17/2024 5:22 PM PST	01/25/2024 8:00 AM PST

Impressions

SCRIPPS RADIOLOGY - 01/25/2024 8:00 AM PST
Normal sinus rhythm
Nonspecific T wave abnormality
Abnormal ECG

Narrative

This result has an attachment that is not available.

Procedure Note

Raghava Gollapudi, MD - 01/25/2024
Formatting of this note might be different from the original.
IMPRESSION:
Normal sinus rhythm
Nonspecific T wave abnormality
Abnormal ECG

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/17/2024 6:43 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/17/2024 9:10 AM PST	

Impressions

01/17/2024 9:11 AM PST
IMPRESSION: Stable exam, overall no significant change

release

Narrative

01/17/2024 9:11 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/17/2024 6:43 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/16/2024 chest x-ray and chest CT

FINDINGS:

Right chest tube in place with ongoing small right effusion. No pneumothorax. Stable cardiac contour and pulmonary vessels. Hazy opacity right base.

Procedure Note

Trevor Nelson, MD - 01/17/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/17/2024 6:43 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/16/2024 chest x-ray and chest CT

FINDINGS:

Right chest tube in place with ongoing small right effusion. No pneumothorax. Stable cardiac contour and pulmonary vessels. Hazy opacity right base.

IMPRESSION:
IMPRESSION: Stable exam, overall no significant change

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/17/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	12.8	3.4 - 11.0 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
RBC	3.36	3.98 - 5.25 M/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
HGB	10.4	11.9 - 15.3 g/dL		01/17/2024 6:07 AM PST	MERCY LAB CV	
HCT	34.4	37.3 - 46.7 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/17/2024 6:07 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/17/2024 6:07 AM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		01/17/2024 6:07 AM PST	MERCY LAB CV	
RDW	14.0	11.7 - 14.9 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Platelet Count	509	150 - 425 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
MPV	9.1	9.0 - 12.8 fL		01/17/2024 6:07 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/17/2024 6:07 AM PST	MERCY LAB CV	
Neutrophils %	86.1	38.0 - 74.0 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
IG%	0.9	0.0 - 1.2 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Lymphocytes %	7.6	16.0 - 48.0 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Monocytes %	4.9	4.9 - 12.5 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Eosinophils %	0.2	0.0 - 9.5 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Basophils %	0.3	0.0 - 1.6 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	11.00	1.50 - 7.40 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
IG#	0.12	0.00 - 0.10 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	0.97	0.90 - 3.10 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.62	0.26 - 0.87 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.03	0.00 - 0.51 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	11.00	1.50 - 7.40 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/17/2024 5:19 AM PST	01/17/2024 5:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/17/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/17/2024 6:19 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Urea Nitrogen	19	7 - 17 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/17/2024 6:19 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Anion Gap	1	6 - 14 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Osmolality Calculated	276	280 - 305 mOsm/kg H20		01/17/2024 6:19 AM PST	MERCY LAB CV	
Calcium	8.6	8.4 - 10.2 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Protein, total	6.1	6.3 - 8.2 g/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Albumin	2.5	3.5 - 5.0 g/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Alkaline Phosphatase	77	38 - 126 Units/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
AST	24	14 - 36 Units/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/17/2024 5:19 AM PST	01/17/2024 5:56 AM PST

Narrative

MERCY LAB CV - 01/17/2024 6:19 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS MICROSCOPIC W/ REFLEX TO CULTURE - Final result (01/16/2024 2:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC (UA)	0-2	0 - 2 / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
RBC (UA)	0-2	0 - 2 / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Epithelial Cells, Squamous (UA)	Few	None / LPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
Bacteria (UA)	Few	None / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
Amorphous Urates Crystals (UA)	Few	None / LPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
Budding Yeast (UA)	Few	None / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/16/2024 2:33 PM PST	01/16/2024 3:28 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS, MACROSCOPIC, W/REFLEX TO CULTURE (URINALYSIS W/REFLEX TO CULTURE) - Final result (01/16/2024 2:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Color (UA)	Yellow	Yellow		01/16/2024 3:43 PM PST	MERCY LAB CV	
Clarity (UA)	Clear	Clear		01/16/2024 3:43 PM PST	MERCY LAB CV	
Specific Gravity (UA)	1.025	1.005 - 1.030		01/16/2024 3:43 PM PST	MERCY LAB CV	
pH (UA)	6.0	5.0 - 8.5 pH		01/16/2024 3:43 PM PST	MERCY LAB CV	
Protein (UA)	30	Negative mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	
Glucose (UA)	Negative	Negative mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	
Ketones (UA)	Negative	Negative mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	
Bilirubin (UA)	Negative	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	
Occult Blood (UA)	Negative	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	
Leukocyte Esterase (UA)	Negative	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Nitrite (UA)	Positive	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	
Urobilinogen (UA)	0.2	<2.0 mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/16/2024 2:33 PM PST	01/16/2024 3:28 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINE CULTURE - Final result (01/16/2024 2:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Urine Culture	Multiple organisms present, probable skin/ urogenital contamination.		MIC	01/18/2024 11:59 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/16/2024 2:33 PM PST	01/16/2024 3:42 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (01/16/2024 11:25 AM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/16/2024 11:52 AM PST	

Impressions

01/16/2024 11:52 AM PST
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Narrative

01/16/2024 11:52 AM PST
EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/16/2024 11:25 AM

HISTORY: RULE OUT DVT

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

Procedure Note

Harold Pimenta, MD - 01/16/2024
Formatting of this note might be different from the original.
EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/16/2024 11:25 AM

HISTORY: RULE OUT DVT

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

IMPRESSION:
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	IMG US PROCEDURES	Final Result

CT chest without contrast (CT CHEST WITHOUT CONTRAST) - Final result (01/16/2024 9:51 AM PST)

Anatomical Region	Laterality	Modality
Chest		Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/16/2024 9:57 AM PST	

Impressions

01/16/2024 10:07 AM PST

IMPRESSION:

1. Small to moderate right pleural effusion with indwelling pleural drainage catheter. Subjacent right basilar atelectasis/consolidation

2. Right upper lobe groundglass opacity with peripheral consolidation (reverse halo sign). Findings can be seen in the setting of atypical community-acquired bacterial pneumonia, fungal pneumonia, and organizing pneumonia.

release

Narrative

01/16/2024 10:07 AM PST

EXAMINATION: CT CHEST WITHOUT CONTRAST

DATE/TIME: 1/16/2024 9:51 AM

HISTORY: 59 years old Female, Pneumonia

COMPARISON: Same day chest radiograph.

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

Lungs: Small to moderate right pleural effusion with indwelling pleural drainage catheter and scattered expected foci of pleural air. Subjacent right basilar atelectasis/consolidation. Right upper lobe groundglass opacity with mild peripheral consolidation (series 5/102). Multifocal right middle and lower lobe scarring and volume loss.

Airways: Patent trachea and central airways. Mild diffuse bronchial wall thickening.

Vasculature: Thoracic aortic atherosclerosis without aneurysm. Main pulmonary artery enlargement up to 3.8 cm.

Cardiac: Mild coronary artery calcifications. Mild cardiomegaly. No pericardial effusion.

Lymph nodes: Limited evaluation without IV contrast. No bulky supraclavicular, axillary, mediastinal, or hilar lymphadenopathy.

Other mediastinal structures: Mild esophageal wall thickening, which may be due to underdistention or esophagitis.

Chest wall: No significant abnormality.

Bones: Old healed midsternal fracture. Degenerative changes of the spine. No aggressive osseous lesion.

Upper abdomen: No significant abnormality.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: Chest; Phantom: 32 cm; CTDIvol: 5; DLP: 177

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

Procedure Note

Pranay Kothari, MD - 01/16/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: CT CHEST WITHOUT CONTRAST

DATE/TIME: 1/16/2024 9:51 AM

HISTORY: 59 years old Female, Pneumonia

COMPARISON: Same day chest radiograph.

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

Lungs: Small to moderate right pleural effusion with indwelling pleural drainage catheter and scattered expected foci of pleural air. Subjacent right basilar atelectasis/consolidation. Right upper lobe groundglass opacity with mild peripheral consolidation (series 5/102). Multifocal right middle and lower lobe scarring and volume loss.

Airways: Patent trachea and central airways. Mild diffuse bronchial wall thickening.

Vasculature: Thoracic aortic atherosclerosis without aneurysm. Main pulmonary artery enlargement up to 3.8 cm.

Cardiac: Mild coronary artery calcifications. Mild cardiomegaly. No pericardial effusion.

Lymph nodes: Limited evaluation without IV contrast. No bulky supraclavicular, axillary, mediastinal, or hilar lymphadenopathy.

Other mediastinal structures: Mild esophageal wall thickening, which may be due to underdistention or esophagitis.

Chest wall: No significant abnormality.

Bones: Old healed midsternal fracture. Degenerative changes of the spine. No aggressive osseous lesion.

Upper abdomen: No significant abnormality.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: Chest; Phantom: 32 cm; CTDIvol: 5; DLP: 177

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

IMPRESSION:
IMPRESSION:

1. Small to moderate right pleural effusion with indwelling pleural drainage catheter. Subjacent right basilar atelectasis/ consolidation

2. Right upper lobe groundglass opacity with peripheral consolidation (reverse halo sign). Findings can be seen in the setting of atypical community-acquired bacterial pneumonia, fungal pneumonia, and organizing pneumonia.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG CT PROCEDURES	Final Result

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/16/2024 6:27 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/16/2024 9:35 AM PST	

Impressions

01/16/2024 9:35 AM PST
IMPRESSION: No significant interval change.

release
CC Dr: p: f:

Narrative

01/16/2024 9:35 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/16/2024 6:27 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/15/2024

FINDINGS: Stable position of the right chest tube. Moderate volume right pleural effusion/thickening is seen with mild to moderate patchy and streak-like change of the right lung base. Stable cardiomegaly with mild bronchovascular prominence. No left lung infiltrate or clear-cut pneumothorax.

Procedure Note

Ross Christensen, MD - 01/16/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/16/2024 6:27 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/15/2024

FINDINGS: Stable position of the right chest tube. Moderate volume right pleural effusion/thickening is seen with mild to moderate patchy and streak-like change of the right lung base. Stable cardiomegaly with mild bronchovascular prominence. No left lung infiltrate or clear-cut pneumothorax.

IMPRESSION:
IMPRESSION: No significant interval change.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/16/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	13.1	3.4 - 11.0 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
RBC	3.37	3.98 - 5.25 M/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HGB	10.6	11.9 - 15.3 g/dL		01/16/2024 5:49 AM PST	MERCY LAB CV	
HCT	35.1	37.3 - 46.7 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
MCV	104	81 - 100 fL		01/16/2024 5:49 AM PST	MERCY LAB CV	
MCH	32	26 - 33 pg		01/16/2024 5:49 AM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		01/16/2024 5:49 AM PST	MERCY LAB CV	
RDW	14.5	11.7 - 14.9 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Platelet Count	546	150 - 425 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
MPV	9.3	9.0 - 12.8 fL		01/16/2024 5:49 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/16/2024 5:49 AM PST	MERCY LAB CV	
Neutrophils %	85.4	38.0 - 74.0 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
IG%	2.0	0.0 - 1.2 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Lymphocytes %	8.1	16.0 - 48.0 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Monocytes %	4.1	4.9 - 12.5 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Eosinophils %	0.2	0.0 - 9.5 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Neutrophils	11.21	1.50 - 7.40 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
IG#	0.26	0.00 - 0.10 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.07	0.90 - 3.10 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Monocytes	0.54	0.26 - 0.87 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.02	0.00 - 0.51 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Basophils	0.03	0.00 - 0.09 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Preliminary Abs Neut	11.21	1.50 - 7.40 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/16/2024 5:19 AM PST	01/16/2024 5:40 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/16/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/16/2024 9:33 AM PST	MERCY LAB CV	
Glucose	116	70 - 125 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/16/2024 9:33 AM PST	MERCY LAB CV	
Anion Gap	-6	6 - 14 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Osmolality Calculated	276	280 - 305 mOsm/kg H20		01/16/2024 9:33 AM PST	MERCY LAB CV	
Calcium	8.5	8.4 - 10.2 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protein, total	6.9	6.3 - 8.2 g/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Alkaline Phosphatase	95	38 - 126 Units/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
AST	102	14 - 36 Units/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Bilirubin, Total	0.4	0.2 - 1.3 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/16/2024 5:19 AM PST	01/16/2024 6:01 AM PST

Narrative

MERCY LAB CV - 01/16/2024 9:33 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/15/2024 11:42 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/15/2024 11:43 AM PST	

Impressions

01/15/2024 11:44 AM PST
IMPRESSION:
1. Worsening right basilar atelectasis/consolidation.
2. Persistent right pleural effusion.
3. Right chest tube is unchanged. No pneumothorax.

release
CC Dr: p: f:

Narrative

01/15/2024 11:44 AM PST

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/15/2024 11:42 AM

HISTORY: Respiratory failure

COMPARISON: 1/14/2024

FINDINGS:
Endotracheal tube has been removed.

Right chest tube is unchanged.

There is no significant pneumothorax.

Increase in right basilar hazy and linear pulmonary opacification. Right costophrenic angle remains blunted.

Left costophrenic angle is sharp.

There is no evidence of pneumothorax.

Cardiomegaly.

Procedure Note

Stephen Hulse, MD - 01/15/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/15/2024 11:42 AM

HISTORY: Respiratory failure

COMPARISON: 1/14/2024

FINDINGS:
Endotracheal tube has been removed.

Right chest tube is unchanged.

There is no significant pneumothorax.

Increase in right basilar hazy and linear pulmonary opacification. Right costophrenic angle remains blunted.

Left costophrenic angle is sharp.

There is no evidence of pneumothorax.

Cardiomegaly.

IMPRESSION:
IMPRESSION:
1. Worsening right basilar atelectasis/consolidation.
2. Persistent right pleural effusion.
3. Right chest tube is unchanged. No pneumothorax.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

GLUCOSE, POC - Final result (01/15/2024 5:52 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	97	70 - 125 mg/dL		01/15/2024 5:54 AM PST	MERCY LAB CV	
Operator ID	233,925			01/15/2024 5:54 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/15/2024 5:52 AM PST	01/15/2024 5:54 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/15/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	12.5	3.4 - 11.0 K/mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
RBC	3.31	3.98 - 5.25 M/mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
HGB	10.4	11.9 - 15.3 g/dL		01/15/2024 5:47 AM PST	MERCY LAB CV	
HCT	33.9	37.3 - 46.7 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/15/2024 5:47 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/15/2024 5:47 AM PST	MERCY LAB CV	
MCHC	30.7	31.0 - 36.0 g/dL		01/15/2024 5:47 AM PST	MERCY LAB CV	
RDW	14.6	11.7 - 14.9 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Platelet Count	543	150 - 425 K/mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
MPV	9.3	9.0 - 12.8 fL		01/15/2024 5:47 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/15/2024 5:47 AM PST	MERCY LAB CV	
Neutrophils %	85.7	38.0 - 74.0 %		01/15/2024 5:47 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
IG%	2.6	0.0 - 1.2 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Lymphocytes %	7.5	16.0 - 48.0 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Monocytes %	3.8	4.9 - 12.5 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Eosinophils %	0.2	0.0 - 9.5 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Neutrophils	10.68	1.50 - 7.40 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
IG#	0.33	0.00 - 0.10 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Lymphocytes	0.93	0.90 - 3.10 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Monocytes	0.48	0.26 - 0.87 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.02	0.00 - 0.51 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Preliminary Abs Neut	10.68	1.50 - 7.40 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/15/2024 5:28 AM PST	01/15/2024 5:43 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (01/15/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/15/2024 6:12 AM PST	MERCY LAB CV	
Glucose	102	70 - 125 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	135	137 - 145 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
CO2, Total	34	22 - 30 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Urea Nitrogen	33	7 - 17 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	
Creatinine	0.90	0.50 - 1.00 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	74	>60 mL/min/1.73m*2		01/15/2024 6:12 AM PST	MERCY LAB CV	
Anion Gap	-1	6 - 14 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Osmolality Calculated	287	280 - 305 mOsm/kg H20		01/15/2024 6:12 AM PST	MERCY LAB CV	
Calcium	8.4	8.4 - 10.2 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/15/2024 5:28 AM PST	01/15/2024 5:44 AM PST

Narrative

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MERCY LAB CV - 01/15/2024 6:12 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/15/2024 12:15 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	103	70 - 125 mg/dL		01/15/2024 12:19 AM PST	MERCY LAB CV	
Operator ID	233,925			01/15/2024 12:19 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/15/2024 12:15 AM PST	01/15/2024 12:19 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Truc Ho MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 5:17 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	93	70 - 125 mg/dL		01/14/2024 5:19 PM PST	MERCY LAB CV	
Operator ID	244,516			01/14/2024 5:19 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 5:17 PM PST	01/14/2024 5:19 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 12:29 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	101	70 - 125 mg/dL		01/14/2024 12:32 PM PST	MERCY LAB CV	
Operator ID	145,602			01/14/2024 12:32 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 12:29 PM PST	01/14/2024 12:32 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 6:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	94	70 - 125 mg/dL		01/14/2024 6:19 AM PST	MERCY LAB CV	
Operator ID	232,690			01/14/2024 6:19 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 6:11 AM PST	01/14/2024 6:19 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	14.3	3.4 - 11.0 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
RBC	3.14	3.98 - 5.25 M/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
HGB	9.7	11.9 - 15.3 g/dL		01/14/2024 5:52 AM PST	MERCY LAB CV	
HCT	30.4	37.3 - 46.7 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
MCV	97	81 - 100 fL		01/14/2024 5:52 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/14/2024 5:52 AM PST	MERCY LAB CV	
MCHC	31.9	31.0 - 36.0 g/dL		01/14/2024 5:52 AM PST	MERCY LAB CV	
RDW	14.6	11.7 - 14.9 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Platelet Count	573	150 - 425 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
MPV	9.6	9.0 - 12.8 fL		01/14/2024 5:52 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
nRBC	0.0	0.0 /100 WBCs		01/14/2024 5:52 AM PST	MERCY LAB CV	
Neutrophils %	84.2	38.0 - 74.0 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
IG%	2.8	0.0 - 1.2 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Lymphocytes %	9.7	16.0 - 48.0 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Monocytes %	3.0	4.9 - 12.5 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Eosinophils %	0.1	0.0 - 9.5 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Neutrophils	12.01	1.50 - 7.40 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
IG#	0.40	0.00 - 0.10 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.39	0.90 - 3.10 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Monocytes	0.43	0.26 - 0.87 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.01	0.00 - 0.51 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Preliminary Abs Neut	12.01	1.50 - 7.40 K/ mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:22 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MAGNESIUM - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Magnesium	2.3	1.6 - 2.3 mg/dL		01/14/2024 5:45 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

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MERCY LAB CV - 01/14/2024 5:45 AM PST

NOTE: Higher values can be expected in females during menses.
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Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CK - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
CK	128	30 - 135 Units/L		01/14/2024 5:45 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

VANCOMYCIN LEVEL, RANDOM - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Vancomycin Level, Random	16	Not Established mcg/mL		01/14/2024 5:50 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/14/2024 7:41 AM PST	MERCY LAB CV	
Glucose	144	70 - 125 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	
Sodium	133	137 - 145 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Chloride	104	98 - 107 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
CO2, Total	26	22 - 30 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Urea Nitrogen	50	7 - 17 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	
Creatinine	1.20	0.50 - 1.00 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	52	>60 mL/min/1.73m*2		01/14/2024 7:41 AM PST	MERCY LAB CV	
Anion Gap	3	6 - 14 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Osmolality Calculated	292	280 - 305 mOsm/kg H20		01/14/2024 7:41 AM PST	MERCY LAB CV	
Calcium	8.1	8.4 - 10.2 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

MERCY LAB CV - 01/14/2024 7:41 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/14/2024 4:29 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/14/2024 9:20 AM PST	

Impressions

01/14/2024 9:22 AM PST
IMPRESSION:
Essentially unchanged examination.

release

Narrative

01/14/2024 9:22 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/14/2024 4:29 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/13/2024

FINDINGS:
One view obtained.

Right basilar atelectasis/consolidation. There is a small right pleural effusion. Right chest tube remains in place. No appreciable pneumothorax. Left lung appears clear.

The cardiomediastinal silhouette is enlarged. Tip of the endotracheal tube projects 4.1 cm superior to the carina. Enteric tube passes below the diaphragm, tip not seen. The osseous structures demonstrate no acute abnormality.

Procedure Note

Anshuman Bansal, MD - 01/14/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/14/2024 4:29 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/13/2024

FINDINGS:
One view obtained.

Right basilar atelectasis/consolidation. There is a small right pleural effusion. Right chest tube remains in place. No appreciable pneumothorax. Left lung appears clear.

The cardiomediastinal silhouette is enlarged. Tip of the endotracheal tube projects 4.1 cm superior to the carina. Enteric tube passes below the diaphragm, tip not seen. The osseous structures demonstrate no acute abnormality.

IMPRESSION:
IMPRESSION:
Essentially unchanged examination.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

BLOOD GAS LACTIC ACID, ARTERIAL - Final result (01/14/2024 3:47 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.45	7.35 - 7.45		01/14/2024 4:06 AM PST	MERCY LAB CV	
pCO2, Arterial	42	35 - 45 mm Hg		01/14/2024 4:06 AM PST	MERCY LAB CV	
pO2, Arterial	56	83 - 108 mm Hg		01/14/2024 4:06 AM PST	MERCY LAB CV	
HCO3, Arterial	29.4	21.0 - 28.0 mmol/L		01/14/2024 4:06 AM PST	MERCY LAB CV	
O2 Sat, Arterial	90.1	95.0 - 98.0 %		01/14/2024 4:06 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	5.5	-2.0 - 3.0 mmol/L		01/14/2024 4:06 AM PST	MERCY LAB CV	
FIO2, Arterial	35.0			01/14/2024 4:06 AM PST	MERCY LAB CV	
Oxygen Device	Ventilator			01/14/2024 4:06 AM PST	MERCY LAB CV	
Modes	PRVC			01/14/2024 4:06 AM PST	MERCY LAB CV	
Lactic Acid, Arterial	1.0	0.4 - 0.9 mmol/L		01/14/2024 4:06 AM PST	MERCY LAB CV	
Allen test	NA			01/14/2024 4:06 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/14/2024 4:06 AM PST	MERCY LAB CV	
Comment	400/20/5			01/14/2024 4:06 AM PST	MERCY LAB CV	
Notified By	243560			01/14/2024 4:06 AM PST	MERCY LAB CV	
Notified Whom	258646			01/14/2024 4:06 AM PST	MERCY LAB CV	
Operator ID	243,560			01/14/2024 4:06 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/14/2024 3:47 AM PST	01/14/2024 4:06 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/14/2024 12:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	173	70 - 125 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Sodium	134	137 - 145 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Chloride	104	98 - 107 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
CO2, Total	28	22 - 30 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Urea Nitrogen	53	7 - 17 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Creatinine	1.22	0.50 - 1.00 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	51	>60 mL/min/1.73m*2		01/14/2024 12:41 AM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Osmolality Calculated	297	280 - 305 mOsm/kg H20		01/14/2024 12:41 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Phosphorus	2.9	2.5 - 4.5 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 12:22 AM PST	01/14/2024 12:26 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 12:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	94	70 - 125 mg/dL		01/14/2024 12:21 AM PST	MERCY LAB CV	
Operator ID	234,267			01/14/2024 12:21 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 12:19 AM PST	01/14/2024 12:21 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

STREP PNEUMO ANTIGEN, URINE - Final result (01/13/2024 11:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Strep Pneumo Ag,UR	Positive	Negative		01/14/2024 3:48 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown	Non-Lab Collection / Unknown	01/13/2024 11:41 PM PST	01/13/2024 11:58 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

LEGIONELLA ANTIGEN URINE (LEGIONELLA ANTIGEN, URINE) - Final result (01/13/2024 11:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Legionella Ag, Urine	Negative	Negative		01/14/2024 3:48 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown	Non-Lab Collection / Unknown	01/13/2024 11:41 PM PST	01/13/2024 11:58 PM PST

Narrative

SCRIPPS LAB MICROBIOLOGY - 01/14/2024 3:48 PM PST
Negative (presumptive) for Legionella pneumophilia, serogroup 1 antigen. Suggests no recent or current infection. Infection cannot be ruled out as antigen may not be present in urine in early infection or the level of antigen may be below detectable limits.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

CULTURE, RESPIRATORY (RESPIRATORY CULTURE WITH STAIN) - Final result (01/13/2024 9:40 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Culture	No Growth Final		MIC	01/16/2024 8:09 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	Moderate WBCs			01/16/2024 8:09 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	No organisms seen			01/16/2024 8:09 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Sputum	Sputum / Unknown	Non-Lab Collection / Unknown	01/13/2024 9:40 PM PST	01/13/2024 9:43 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/13/2024 8:28 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	135	70 - 125 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Potassium	4.7	3.5 - 5.1 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
CO2, Total	27	22 - 30 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Urea Nitrogen	56	7 - 17 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Creatinine	1.30	0.50 - 1.00 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	47	>60 mL/min/1.73m*2		01/13/2024 8:50 PM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Osmolality Calculated	292	280 - 305 mOsm/kg H20		01/13/2024 8:50 PM PST	MERCY LAB CV	
Calcium	8.0	8.4 - 10.2 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Phosphorus	3.5	2.5 - 4.5 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 8:28 PM PST	01/13/2024 8:31 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/13/2024 6:07 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	99	70 - 125 mg/dL		01/13/2024 6:09 PM PST	MERCY LAB CV	
Operator ID	155,359			01/13/2024 6:09 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 6:07 PM PST	01/13/2024 6:09 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BETA HYDROXYBUTYRATE - Final result (01/13/2024 3:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Beta-Hydroxybutyrate	2.2	0.2 - 2.8 mg/dL		01/13/2024 3:59 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 3:33 PM PST	01/13/2024 3:36 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

VANCOMYCIN LEVEL, RANDOM - Final result (01/13/2024 3:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Vancomycin Level, Random	14	Not Established mcg/mL		01/13/2024 3:56 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 3:33 PM PST	01/13/2024 3:36 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (01/13/2024 3:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/13/2024 3:51 PM PST	MERCY LAB CV	
Glucose	132	70 - 125 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	131	137 - 145 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Potassium	5.3	3.5 - 5.1 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
CO2, Total	25	22 - 30 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Urea Nitrogen	59	7 - 17 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	
Creatinine	1.20	0.50 - 1.00 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	52	>60 mL/min/1.73m*2		01/13/2024 3:51 PM PST	MERCY LAB CV	
Anion Gap	6	6 - 14 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Osmolality Calculated	290	280 - 305 mOsm/kg H20		01/13/2024 3:51 PM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 3:33 PM PST	01/13/2024 3:36 PM PST

Narrative

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MERCY LAB CV - 01/13/2024 3:51 PM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

Ultrasound abdominal limited liver (ULTRASOUND ABDOMEN LIMITED LIVER) - Final result (01/13/2024 2:52 PM PST)

Anatomical Region	Laterality	Modality
Body		Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 2:54 PM PST	

Impressions

01/13/2024 2:55 PM PST

IMPRESSION: Unremarkable gallbladder.

Echogenic liver. Consider steatosis or hepatocellular disease.

Right pleural effusion

release

Narrative

01/13/2024 2:55 PM PST

EXAMINATION: ULTRASOUND ABDOMEN LIMITED LIVER

DATE/TIME: 1/13/2024 2:52 PM

HISTORY: Reported abdominal pain, please also evaluate gallbladder

COMPARISON: None.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: Length is 19.1 cm. Increased echogenicity. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
BILIARY AND GALLBLADDER: Normal gallbladder. No ductal dilatation. CBD diameter is 5.1 mm.
PANCREAS: Not well visualized
RIGHT KIDNEY: Right renal length is 10.0 cm. No visible solid mass, calculus or hydronephrosis.
AORTA/IVC/VASC: No visible aneurysm. The intrahepatic IVC is patent
OTHER: Right pleural effusion

Procedure Note

Raymond Hong, MD - 01/13/2024

Formatting of this note might be different from the original.

EXAMINATION: ULTRASOUND ABDOMEN LIMITED LIVER

DATE/TIME: 1/13/2024 2:52 PM

HISTORY: Reported abdominal pain, please also evaluate gallbladder

COMPARISON: None.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: Length is 19.1 cm. Increased echogenicity. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
BILIARY AND GALLBLADDER: Normal gallbladder. No ductal dilatation. CBD diameter is 5.1 mm.
PANCREAS: Not well visualized
RIGHT KIDNEY: Right renal length is 10.0 cm. No visible solid mass, calculus or hydronephrosis.
AORTA/IVC/VASC: No visible aneurysm. The intrahepatic IVC is patent
OTHER: Right pleural effusion

IMPRESSION:
IMPRESSION: Unremarkable gallbladder.

Echogenic liver. Consider steatosis or hepatocellular disease.

Right pleural effusion

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG US PROCEDURES	Final Result

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/13/2024 12:38 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	347	70 - 125 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Potassium	5.8	3.5 - 5.1 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
CO2, Total	20	22 - 30 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Urea Nitrogen	55	7 - 17 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Creatinine	1.10	0.50 - 1.00 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	58	>60 mL/min/1.73m*2		01/13/2024 1:14 PM PST	MERCY LAB CV	
Anion Gap	9	6 - 14 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Osmolality Calculated	301	280 - 305 mOsm/kg H20		01/13/2024 1:14 PM PST	MERCY LAB CV	
Calcium	7.9	8.4 - 10.2 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Phosphorus	5.7	2.5 - 4.5 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 12:38 PM PST	01/13/2024 12:48 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/13/2024 12:09 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 12:16 PM PST	

Impressions

01/13/2024 12:17 PM PST
IMPRESSION: Interval decrease in moderate right pleural effusion status post chest tube insertion. No pneumothorax.

release

Narrative

01/13/2024 12:17 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 12:09 PM

HISTORY: s/p chest tube insertion

COMPARISON: 1/13/2024

FINDINGS:

New right chest tube. Endotracheal and gastric tubes are in stable position.

The heart and mediastinum are unchanged.

Interval decrease in moderate right pleural effusion. No pneumothorax. Right basilar atelectasis or infiltrate again noted.

Imaged skeletal structures are unchanged.

Procedure Note

Raymond Hong, MD - 01/13/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 12:09 PM

HISTORY: s/p chest tube insertion

COMPARISON: 1/13/2024

FINDINGS:

New right chest tube. Endotracheal and gastric tubes are in stable position.

The heart and mediastinum are unchanged.

Interval decrease in moderate right pleural effusion. No pneumothorax. Right basilar atelectasis or infiltrate again noted.

Imaged skeletal structures are unchanged.

IMPRESSION:
IMPRESSION: Interval decrease in moderate right pleural effusion status post chest tube insertion. No pneumothorax.

release

Authorizing Provider	Result Type	Result Status
Brandon May DNP	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

GLUCOSE, POC - Final result (01/13/2024 12:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	111	70 - 125 mg/dL		01/13/2024 4:12 PM PST	MERCY LAB CV	
Operator ID	244,516			01/13/2024 4:12 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 12:03 PM PST	01/13/2024 4:12 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CHEST TUBE INSERTION (01/13/2024 11:44 AM PST)

Narrative

Pedro Dammert Coronado, MD - 01/13/2024 11:44 AM PST

Narrative

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May, Brandon Michael, DNP     1/13/2024 11:46 AM
Chest Tube Insertion

Date/Time: 1/13/2024 11:44 AM

Performed by: May, Brandon Michael, DNP
Authorized by: May, Brandon Michael, DNP
Consent:
 Consent obtained:  Written
 Consent given by:  Healthcare agent
 Risks, benefits, and alternatives were discussed: yes
 Risks discussed:  Bleeding, damage to surrounding structures, nerve
damage, pain, infection and incomplete drainage
 Alternatives discussed:  No treatment
Universal protocol:
 Procedure explained and questions answered to patient or proxy's
satisfaction: yes
 Relevant documents present and verified: yes
 Test results available: yes
 Imaging studies available: yes
 Required blood products, implants, devices, and special equipment
available: yes
 Site/side marked: yes
 Immediately prior to procedure, a time out was called: yes
 Patient identity confirmed:  Hospital-assigned identification number and
arm band
Pre-procedure details:
 Skin preparation:  Chlorhexidine with alcohol
 Preparation: Patient was prepped and draped in the usual sterile fashion

Anesthesia:
 Anesthesia method:  Local infiltration
 Local anesthetic:  Lidocaine 1% w/o epi
Procedure details:
 Placement location:  R lateral
 Tube size (French): 14.
 Ultrasound guidance: yes
 Tension pneumothorax: no
 Tube connected to:  Water seal
 Drainage characteristics:  Purulent and yellow
 Suture material:  2-0 silk
 Dressing: Tegaderm.
Post-procedure details:
 Procedure completion:  Tolerated well, no immediate complications
Comments:
  CXR pending, Dr. Dammert at bedside for the entire procedure
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Authorizing Provider	Result Type	Result Status
Brandon May DNP	PROCEDURE/MINOR SURGICAL ORDERABLES	Final Result

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (01/13/2024 10:57 AM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 11:27 AM PST	

Impressions

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01/13/2024 11:27 AM PST
IMPRESSION:  Negative for deep vein thrombosis in both lower extremities.

1.7 cm right popliteal cyst


release
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Narrative

01/13/2024 11:27 AM PST

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/13/2024 10:57 AM

HISTORY: Asymmetric edema R>L

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: 1.7 cm right popliteal cyst.

Procedure Note

Raymond Hong, MD - 01/13/2024

Formatting of this note might be different from the original.

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/13/2024 10:57 AM

HISTORY: Asymmetric edema R>L

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: 1.7 cm right popliteal cyst.

IMPRESSION:
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

1.7 cm right popliteal cyst

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG US PROCEDURES	Final Result

2D ECHO COMPLETE WITHOUT CONTRAST (WITH COLOR AND SPECTRAL DOPPLER AND MYOCARDIAL STRAIN) (01/13/2024 9:44 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Interventricular Septal Thickness End Diastole	1.1	cm			SCRIPPS RADIOLOGY	
Left Ventricular Internal Diameter End Diastole	6.2	cm			SCRIPPS RADIOLOGY	
Left Ventricular Internal Diameter End Systole	5.4	cm			SCRIPPS RADIOLOGY	
Left Ventricular Posterior Wall	1.1	cm			SCRIPPS RADIOLOGY	
Left Ventricular End Systolic Volume Indexed	94	ml			SCRIPPS RADIOLOGY	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Left Ventricular Ejection Fraction	26.2	%			SCRIPPS RADIOLOGY	
Mitral Valve E/A Ratio	0.89				SCRIPPS RADIOLOGY	
Aortic Valve Maximum Velocity	146	cm/sec			SCRIPPS RADIOLOGY	
Aortic Valve Mean Pressure Gradient	4	mmHg			SCRIPPS RADIOLOGY	
Aortic Valve Area	2	cm^2			SCRIPPS RADIOLOGY	
LVOT VTI	18.2	cm			SCRIPPS RADIOLOGY	
Mitral Regurgitation Effective Regurgitant Orifice Area	0.13	cm^2			SCRIPPS RADIOLOGY	
Mitral Regurgitant Volume	20.9	ml			SCRIPPS RADIOLOGY	
Stroke Volume (LVOT) Indexed	26.7	ml/m^2			SCRIPPS RADIOLOGY	
RA Volume (Indexed)	36.3	ml/m^2			SCRIPPS RADIOLOGY	
Mitral Valve Average E/e' Ratio	13.2				SCRIPPS RADIOLOGY	
Left Ventricular End Diastolic Volume Indexed	73.3	ml/m^2			SCRIPPS RADIOLOGY	
Left Atrial Volume Indexed	38.3				SCRIPPS RADIOLOGY	
Ascending Aorta Indexed	2.1	cm			SCRIPPS RADIOLOGY	
Ao Sinus Indexed	2.2	cm			SCRIPPS RADIOLOGY	
Right Ventricular Diameter 2 (mid-level)	1.6	cm			SCRIPPS RADIOLOGY	
TAPSE	1.6	cm			SCRIPPS RADIOLOGY	
Dimensionless Index (LVOT/AoV)	0.81				SCRIPPS RADIOLOGY	

Anatomical Region	Laterality	Modality
		Echocardiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 8:39 AM PST	

Impressions

01/14/2024 7:37 AM PST

Study ID: 1074047

Scripps Mercy Hospital Chula Vista
Cardiology Lab
435 H Street
Chula Vista, CA 91910
(619) 691-7499

Adult Echo Report

Name: PETRUCCIO, ELIZABETH TINA
MRN: 810760324
DOB: 07/29/1964
Age: 59 yrs
Gender: Female
HR: 66
Study Date: 01/13/2024 08:39 AM
Patient Location: SVICU^OICU^11^CV
Height: 67 in
Weight: 140 lb
BSA: 1.7 m2
BP: 105/83 mmHg

Reason For Study: Other Cardiac Reason, Specify in Comments
History: Heart failure reduced ejection fraction on echo, reported history of asthma; medical noncompliance, past alcohol and methamphetamine abuse
Acct Number: 10143864623
Accession Number: 3011620730

Impressions

EXAM/PROCEDURE
Complete 2D/M-mode, color Doppler, and spectral Doppler echocardiogram performed. The study was technically adequate. No prior exam for comparison. Critical value: new LV systolic dysfunction, EF <35%. Sonographer communicated preliminary findings to Dr Damert Coronado at 09:44 on 01/13/2024.

Interpretation Summary
The left ventricle is severely dilated.
Global left ventricular systolic function is severely reduced.

LEFT VENTRICLE
The left ventricle is severely dilated.LVIDd = 6.2 cm. There is increased left ventricular mass with normal relative wall thickness consistent with severe eccentric hypertrophy. LV mass index = 168.0 grams/m^2. Relative wall thickness= 0.36 (nl <0.42).The ejection fraction = 26.2 % by biplane Method of Discs. Ejection Fraction = 25.5 %. Global longitudinal strain is -6.9%. Global left ventricular systolic function is severely reduced. There is no thrombus visualized. A false chord is noted (normal variant).

DIASTOLOGY/HEMODYNAMICS
There is Stage I impaired LV diastolic filling pattern. Avg. E/e' = 13.2 (nl <13). There is evidence for elevated left atrial pressure. Cardiac output is 3.4 l/min (nl 4-7l/min). Cardiac index is 1.9 l/min/m^2 (nl 2.5-4.5l/min/m2). Unable to estimate pulmonary arterial
pressure. Right atrial pressure is estimated as 15mmHg.

RIGHT VENTRICLE
The right ventricle is dilated. RVDd2 = 1.6 cm. RVDd1 = 4.6 cm. The right ventricular ejection fraction is mildly decreased. The right ventricular tricuspid annular plane systolic excursion (TAPSE) measures 1.6 cm.

ATRIA
The left atrium is mildly dilated. Left atrial volume (bi-plane) = 38.3 ml/m2 (nl < 35ml/m2). Right atrium is dilated. Right atrial volume indexed = 36.3 ml/m^2 (nl male/female <39/<34 ml/m^2). There is no Doppler evidence for a patent foramen ovale.

MITRAL VALVE
The mitral regurgitant jet is eccentrically directed. There is mild to moderate mitral regurgitation. Effective regurgitant orifice 0.13 cm^2 (severe >0.4cm2). Mitral regurgitation PISA = 0.50 cm. Regurgitant volume 20.9 ml (severe >/= 60ml/beat).

AORTIC VALVE
Normal trileaflet aortic valve. Trace aortic regurgitation.

TRICUSPID VALVE
The tricuspid valve is normal. There is physiologic tricuspid regurgitation.

PULMONIC VALVE
The pulmonic valve leaflets are thin and pliable; valve motion is normal. Trace pulmonic valvular regurgitation.

AORTA/PULMONARY ARTERY
The aortic sinus of valsalva is dilated. The Aortic Sinus(es) of Valsalva measures 3.8 cm. Indexed Aortic Sinus(es) of Valsalva are 2.2 cm/m2. (normal <=2.1cm/m2). The ascending aorta is dilated. The ascending aorta measures 3.7 cm. Indexed ascending aorta is 2.1 cm/m2. (normal <=1.9cm/m2). The pulmonary artery is dilated. The pulmonary artery measures 3.9 cm.

PERICARDIUM/PLEURAL
There is no pericardial effusion. Large right pleural effusion. The left lung base image was technically limited. A unilateral comet-tail artifact is visualized when imaging the anteroapex of the lung.

MMode/2D Measurements & Calculations

RVDd1(nl <4.2cm): 4.6 cm	LVIDd (3.8-5.8): 6.2 cm
RVDd2(nl <3.6cm): 1.6 cm	LVIDs (2.3-3.9): 5.4 cm
IVSd (0.6-1.1): 1.1 cm	LVPWd (0.6-1.1): 1.1 cm
LV mass(C)d: 291.9 grams	Ao sinus diam: 3.8 cm
LV mass idx (F<96,M<116g/m2): 168.0 grams/m2	AO sinus indexed: 2.2 cm
Relative Wall Thickness: 0.36	asc Aorta Diam: 3.7 cm
	asc AO indexed: 2.1 cm
	LA dimension (1.9-4.0): 3.9 cm
LVOT diam: 1.8 cm	
	LA area AP2: 26.1 cm2
LVOT area: 2.5 cm2	LA area AP4: 18.3 cm2
	LA shortest length: 6.1 cm
	LA VOLUME (indexed): 38.3
MPA diam: 3.9 cm	

Impressions

EDV(MOD-bp): 127.4 ml

EDV(MOD-bp) indexed: 73.3 ml/m2
ESV(MOD-bp): 94.0 ml
EF(MOD-bp): 26.2 %
SV(MOD-bp): 33.3 ml

RA area AP4: 17.7 cm2
RA length: 5.0 cm
RA Volume Indexed: 36.3 ml/m2
TAPSE(nl >=1.7cm): 1.6 cm

Time Measurements
Aortic R-R: 0.82 sec
Aortic HR: 73.0 BPM

Doppler Measurements & Calculations
MV E max vel: 46.0 cm/sec
MV A max vel: 51.8 cm/sec
MV E/A: 0.89
MV dec slope: 206.0 cm/sec2
MV dec time: 0.22 sec
Lat Peak E' Vel: 3.5 cm/sec
Med Peak E' Vel: 3.5 cm/sec
Lateral E/e' ratio: 13.2
Average E/e' ratio: 13.2

Ao V2 max: 146.0 cm/sec
Ao max PG: 8.5 mmHg
Ao V2 mean: 99.4 cm/sec
Ao mean PG: 4.0 mmHg
Ao V2 VTI: 22.6 cm
AVA(I,D): 2.0 cm2

AVA (I,D) indexed for BSA: 1.2 cm2
Dimensionless Index (LV/AV): 0.81
LV V1 max PG: 4.3 mmHg
LV V1 mean PG: 2.0 mmHg
LV V1 max: 104.0 cm/sec
LV V1 mean: 74.5 cm/sec
LV V1 VTI: 18.2 cm

MR max vel: 451.0 cm/sec
MR max PG: 81.4 mmHg
MR mean vel: 355.0 cm/sec
MR mean PG: 55.0 mmHg
MR VTI: 156.0 cm
MR PISA radius: 0.50 cm
MR alias vel: 38.5 cm/sec
MR ERO: 0.13 cm2
MR volume: 20.9 ml
MR flow rate: 60.5 cm3/sec
CO(LVOT): 3.4 l/min
CI(LVOT): 1.9 l/min/m2
SV(LVOT): 46.3 ml
SI(LVOT): 26.7 ml/m2

PA V2 max: 73.0 cm/sec
PA max PG: 2.0 mmHg
RV V1 max: 56.1 cm/sec
RV V1 max PG: 1.3 mmHg
RV S'(nl>=9.5cm/sec): 12.4 cm/sec

MR regurgitant fraction: 31.1 %

Reading Physician:Electronically signed by: Arvin Narula, MD on 01/14/2024 07:37 AM
Ordering Physician: JONES, CHRISTOPHER
Performed By: Rodil, Alla, RDCS

Narrative

This result has an attachment that is not available.

Procedure Note

Arvin Narula, MD - 01/14/2024

Formatting of this note might be different from the original.

IMPRESSION:

Study ID: 1074047

Scripps Mercy Hospital Chula Vista
Cardiology Lab
435 H Street
Chula Vista, CA 91910
(619) 691-7499
Adult Echo Report
Name: PETRUCCIO, ELIZABETH TINA Study Date: 01/13/2024 08:39 AM
MRN: 810760324 Patient Location: SVICU^0ICU^11^CV

Procedure Note

DOB: 07/29/1964 Height: 67 in
Age: 59 yrs Weight: 140 lb
Gender: Female BSA: 1.7 m2
HR: 66 BP: 105/83 mmHg
Reason For Study: Other Cardiac Reason, Specify in Comments
History: Heart failure reduced ejection fraction on echo, reported history of asthma; medical noncompliance, past alcohol and methamphetamine abuse
Acct Number: 10143864623 Accession Number: 3011620730

EXAM/PROCEDURE
Complete 2D/M-mode, color Doppler, and spectral Doppler echocardiogram performed. The study was technically adequate. No prior exam for comparison. Critical value: new LV systolic dysfunction, EF <35%. Sonographer communicated preliminary findings to Dr Damert Coronado at 09:44 on 01/13/2024.

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MV dec slope: 206.0 cm/sec2 Average E/e' ratio: 13.2
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Ao V2 max: 146.0 cm/sec LV V1 max PG: 4.3 mmHg
Ao max PG: 8.5 mmHg LV V1 mean PG: 2.0 mmHg
Ao V2 mean: 99.4 cm/sec LV V1 max: 104.0 cm/sec
Ao mean PG: 4.0 mmHg LV V1 mean: 74.5 cm/sec
Ao V2 VTI: 22.6 cm LV V1 VTI: 18.2 cm
AVA(I,D): 2.0 cm2

AVA (I,D) indexed for BSA: 1.2 cm2
Dimensionless Index (LV/AV): 0.81

MR max vel: 451.0 cm/sec CO(LVOT): 3.4 l/min
MR max PG: 81.4 mmHg CI(LVOT): 1.9 l/min/m2
MR mean vel: 355.0 cm/sec SV(LVOT): 46.3 ml
MR mean PG: 55.0 mmHg SI(LVOT): 26.7 ml/m2
MR VTI: 156.0 cm
MR PISA radius: 0.50 cm
MR alias vel: 38.5 cm/sec
MR ERO: 0.13 cm2
MR volume: 20.9 ml
MR flow rate: 60.5 cm3/sec

PA V2 max: 73.0 cm/sec RV V1 max: 56.1 cm/sec

Procedure Note

PA max PG: 2.0 mmHg RV V1 max PG: 1.3 mmHg
RV S'(nl>=9.5cm/sec): 12.4 cm/sec

MR regurgitant fraction: 31.1 %

Reading Physician:Electronically signed by: Arvin Narula, MD on 01/14/2024 07:37 AM
Ordering Physician: JONES, CHRISTOPHER
Performed By: Rodil, Alla, RDCS

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	CV ECHO PROCEDURES	Edited Result - Final

BLOOD GAS ALL ANALYTES, ARTERIAL - Final result (01/13/2024 7:12 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.40	7.35 - 7.45		01/13/2024 7:16 AM PST	MERCY LAB CV	
pCO2, Arterial	42	35 - 45 mm Hg		01/13/2024 7:16 AM PST	MERCY LAB CV	
pO2, Arterial	286	83 - 108 mm Hg		01/13/2024 7:16 AM PST	MERCY LAB CV	
HCO3, Arterial	25.8	21.0 - 28.0 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
O2 Sat, Arterial	100.0	95.0 - 98.0 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	1.0	-2.0 - 3.0 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
FIO2, Arterial	100.0			01/13/2024 7:16 AM PST	MERCY LAB CV	
Oxygen Device	Ventilator			01/13/2024 7:16 AM PST	MERCY LAB CV	
Modes	PRVC			01/13/2024 7:16 AM PST	MERCY LAB CV	
Total Hgb, Whole Blood	11.1	11.9 - 15.3 g/dL		01/13/2024 7:16 AM PST	MERCY LAB CV	
Oxyhemoglobin, Arterial	98.4	95.0 - 98.0 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Carboxyhemoglobin, Arterial	0.4	1.0 - 1.5 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Methemoglobin, Arterial	1.2	<3.0 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Lactic Acid, Arterial	1.1	0.4 - 0.9 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose, Whole Blood	186	70 - 125 mg/dL		01/13/2024 7:16 AM PST	MERCY LAB CV	
Ionized Calcium, Whole Blood	1.17	1.15 - 1.27 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Sodium, Whole Blood	134	136 - 146 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Potassium, Whole Blood	5.3	3.5 - 5.1 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Chloride, Whole Blood	98	98 - 107 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Allen test	Positive			01/13/2024 7:16 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/13/2024 7:16 AM PST	MERCY LAB CV	
Comment	400/20/+8			01/13/2024 7:16 AM PST	MERCY LAB CV	
Operator ID	243,099			01/13/2024 7:16 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/13/2024 7:12 AM PST	01/13/2024 7:16 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/13/2024 6:59 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 1:42 PM PST	

Impressions

01/13/2024 1:43 PM PST
IMPRESSION: Endotracheal and gastric tubes as described below.

Unchanged large right pleural effusion

release

Narrative

01/13/2024 1:43 PM PST

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 6:59 AM

HISTORY: intubation

COMPARISON: 1/13/2024

FINDINGS:

Endotracheal tube terminates 4.6 cm above the carina. Gastric tube terminates in the proximal stomach.

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis or infiltrate. No pneumothorax.

Imaged skeletal structures are unchanged.

Procedure Note

Raymond Hong, MD - 01/13/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 6:59 AM

HISTORY: intubation

COMPARISON: 1/13/2024

FINDINGS:

Endotracheal tube terminates 4.6 cm above the carina. Gastric tube terminates in the proximal stomach.

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis or infiltrate. No pneumothorax.

Imaged skeletal structures are unchanged.

IMPRESSION:
IMPRESSION: Endotracheal and gastric tubes as described below.

Unchanged large right pleural effusion

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

GLUCOSE, POC - Final result (01/13/2024 6:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	142	70 - 125 mg/dL		01/13/2024 6:42 AM PST	MERCY LAB CV	
Operator ID	157,150			01/13/2024 6:42 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 6:40 AM PST	01/13/2024 6:42 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LIPID PANEL W/ REFLEX TO DIRECT LDL IF INDICATED - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/13/2024 1:02 PM PST	SCRIPPS LAB	
Triglycerides	72	7 - 150 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Comment:

```
Normal          <150      mg/dL
Borderline High  150-199  mg/dL
High             200-499  mg/dL
Very high       >=500     mg/dL

N-Acetyl-L-Cysteine at therapeutically achieved concentrations may lead to falsely low results.
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cholesterol	110	7 - 200 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Comment:

```
Desirable        <200       mg/dL
Borderline high   200-239   mg/dL
High             >= 240     mg/dL
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HDL Cholesterol	33	>=40 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Comment:

```
Major Risk Factor for Heart Disease
<40 mg/dL

Negative Risk Factor for Heart Disease
>= 60 mg/dL

N-acetyl-L-cysteine at elevated concentrations may lead to falsely low results
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
LDL Calculation	63	<100 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

SCRIPPS LAB - 01/13/2024 1:02 PM PST

Narrative

Reference ranges are derived from the clinical practice ATPIII Guidelines of the National Cholesterol Education Program (NCEP) on the Management of Blood Cholesterol: A Report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines, More recent guidelines using clinical and laboratory risk stratification approach provide more specific recommendations for specific patient groups (November 2018).

The relationship between levels of LDL cholesterol and Coronary Heart Disease is continuous over a broad range of LDL levels. A reduction in Cholesterol, Triglycerides and LDL levels has been associated with a reduced risk of CHD.

https://www.ahajournals.org/doi/10.1161/CIR.0000000000000625

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

HIV ANTIGEN/ANTIBODY COMBO - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HIV Antigen / Antibody Combo	Non Reactive	Non Reactive		01/13/2024 12:02 PM PST	SCRIPPS LAB	

Comment: HIV-1 p24 antigen and HIV-1/HIV-2 antibodies are not detected.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:24 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

HEMOGRAM - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	21.4	3.4 - 11.0 K/mcL		01/13/2024 5:34 AM PST	MERCY LAB CV	
RBC	3.83	3.98 - 5.25 M/mcL		01/13/2024 5:34 AM PST	MERCY LAB CV	
HGB	11.8	11.9 - 15.3 g/dL		01/13/2024 5:34 AM PST	MERCY LAB CV	
HCT	38.8	37.3 - 46.7 %		01/13/2024 5:34 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		01/13/2024 5:34 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/13/2024 5:34 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCHC	30.4	31.0 - 36.0 g/dL		01/13/2024 5:34 AM PST	MERCY LAB CV	
RDW	14.9	11.7 - 14.9 %		01/13/2024 5:34 AM PST	MERCY LAB CV	
Platelet Count	801	150 - 425 K/mcL		01/13/2024 5:34 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:24 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.014	<=0.034 ng/mL		01/13/2024 6:00 AM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population. This patient's result is considered normal. However, clinical
correlation is suggested to determine whether serial measurements are necessary for further evaluation.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CK - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
CK	148	30 - 135 Units/L		01/13/2024 5:55 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	235	70 - 125 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Sodium	133	137 - 145 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Potassium	6.0	3.5 - 5.1 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
CO2, Total	28	22 - 30 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Urea Nitrogen	55	7 - 17 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Creatinine	1.39	0.50 - 1.00 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	44	>60 mL/min/1.73m*2		01/13/2024 6:01 AM PST	MERCY LAB CV	
Anion Gap	4	6 - 14 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Osmolality Calculated	299	280 - 305 mOsm/kg H20		01/13/2024 6:01 AM PST	MERCY LAB CV	
Calcium	8.7	8.4 - 10.2 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Phosphorus	9.4	2.5 - 4.5 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Albumin	3.4	3.5 - 5.0 g/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BLOOD GAS, ARTERIAL - Final result (01/13/2024 5:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.10	7.35 - 7.45		01/13/2024 5:09 AM PST	MERCY LAB CV	
pCO2, Arterial	87	35 - 45 mm Hg		01/13/2024 5:09 AM PST	MERCY LAB CV	
pO2, Arterial	117	83 - 108 mm Hg		01/13/2024 5:09 AM PST	MERCY LAB CV	
HCO3, Arterial	27.2	21.0 - 28.0 mmol/L		01/13/2024 5:09 AM PST	MERCY LAB CV	
O2 Sat, Arterial	97.0	95.0 - 98.0 %		01/13/2024 5:09 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	-2.4	-2.0 - 3.0 mmol/L		01/13/2024 5:09 AM PST	MERCY LAB CV	
FIO2, Arterial	60.0			01/13/2024 5:09 AM PST	MERCY LAB CV	
Oxygen Device	BIPAP			01/13/2024 5:09 AM PST	MERCY LAB CV	
Modes	NA			01/13/2024 5:09 AM PST	MERCY LAB CV	
Allen test	NA			01/13/2024 5:09 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/13/2024 5:09 AM PST	MERCY LAB CV	
Comment	14/7/16			01/13/2024 5:09 AM PST	MERCY LAB CV	
Notified By	243560			01/13/2024 5:09 AM PST	MERCY LAB CV	
Notified Whom	157150			01/13/2024 5:09 AM PST	MERCY LAB CV	
Operator ID	243,560			01/13/2024 5:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/13/2024 5:01 AM PST	01/13/2024 5:09 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/13/2024 4:09 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 12:26 PM PST	

Impressions

01/13/2024 12:27 PM PST
IMPRESSION: No significant change.

release

Narrative

01/13/2024 12:27 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 4:09 AM

HISTORY: f/u thoracentesis

COMPARISON: 1/12/2024

FINDINGS:

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis. No pneumothorax.

Imaged skeletal structures are unchanged.

Procedure Note

Raymond Hong, MD - 01/13/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 4:09 AM

HISTORY: f/u thoracentesis

COMPARISON: 1/12/2024

FINDINGS:

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis. No pneumothorax.

Imaged skeletal structures are unchanged.

IMPRESSION:
IMPRESSION: No significant change.

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

BLOOD GAS LACTIC ACID, ARTERIAL - Final result (01/13/2024 2:55 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.13	7.35 - 7.45		01/13/2024 3:11 AM PST	MERCY LAB CV	
pCO2, Arterial	84	35 - 45 mm Hg		01/13/2024 3:11 AM PST	MERCY LAB CV	
pO2, Arterial	78	83 - 108 mm Hg		01/13/2024 3:11 AM PST	MERCY LAB CV	
HCO3, Arterial	28.2	21.0 - 28.0 mmol/L		01/13/2024 3:11 AM PST	MERCY LAB CV	
O2 Sat, Arterial	90.5	95.0 - 98.0 %		01/13/2024 3:11 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	-1.0	-2.0 - 3.0 mmol/L		01/13/2024 3:11 AM PST	MERCY LAB CV	
FIO2, Arterial	32.0			01/13/2024 3:11 AM PST	MERCY LAB CV	
Oxygen Device	NC			01/13/2024 3:11 AM PST	MERCY LAB CV	
Modes	NA			01/13/2024 3:11 AM PST	MERCY LAB CV	
Lactic Acid, Arterial	1.1	0.4 - 0.9 mmol/L		01/13/2024 3:11 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Allen test	NA			01/13/2024 3:11 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/13/2024 3:11 AM PST	MERCY LAB CV	
Comment	4L NC			01/13/2024 3:11 AM PST	MERCY LAB CV	
Notified By	243560			01/13/2024 3:11 AM PST	MERCY LAB CV	
Notified Whom	157150			01/13/2024 3:11 AM PST	MERCY LAB CV	
Operator ID	243,560			01/13/2024 3:11 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/13/2024 2:55 AM PST	01/13/2024 3:11 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PATHOLOGIST FLUID COMMENT - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Pathologist Review, CSF/ Body Fluids	Morphologic analysis of the smear shows predominantly acute inflammation. Yi Zhuang, M.D.			01/15/2024 9:24 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:07 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, WOUND (WOUND/FLUID/TISSUE CULTURE W/STAIN) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Wound Culture	No Growth Final		MIC	01/17/2024 8:53 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	Many WBCs			01/17/2024 8:53 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	No organisms seen			01/17/2024 8:53 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

BODY FLUID CELL COUNT WITH DIFF (BODY FLUID CELL COUNT WITH DIFFERENTIAL) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Volume (ML)	17.0	mL		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Color	Light Yellow			01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Appearance	Cloudy			01/13/2024 1:20 PM PST	MERCY LAB CV	
Comment: Cellular debris present may affect results.						
Fluid Nucleated Cell Count	103,752	/mcL		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid RBC Count	20,000	/mcL		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Neutrophils %	94	%		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Lymphocytes %	1	%		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Mononuclears %	5	%		01/13/2024 1:20 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Bacteria	Present	None Seen		01/13/2024 1:20 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:07 AM PST

Narrative

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MERCY LAB CV - 01/13/2024 1:20 PM PST
Reference Interval has not been established for this specimen type.
```

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTEIN BODY FLUID (PROTEIN, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/13/2024 3:48 AM PST	MERCY LAB CV	
Protein (FL)	4.5	Not Established g/dL		01/13/2024 3:48 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

```
MERCY LAB CV - 01/13/2024 3:48 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories.
The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for clinical purposes.
```

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTATE DEHYDROGENASE BODY FLUID (LACTATE DEHYDROGENASE, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/13/2024 4:06 AM PST	MERCY LAB CV	
Lactate Dehrdrogenase (FL)	1,379	Not Established Units/L		01/13/2024 4:06 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

```
MERCY LAB CV - 01/13/2024 4:06 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories.
The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for clinical purposes.
```

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE BODY FLUID (GLUCOSE, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/13/2024 3:48 AM PST	MERCY LAB CV	
Glucose (FL)	<20	Not Established mg/dL		01/13/2024 3:48 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

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MERCY LAB CV - 01/13/2024 3:48 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories.
The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for clinical purposes.
```

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

ALBUMIN BODY FLUID (ALBUMIN, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Albumin (FL)	2.0	Not Established g/dL		01/13/2024 3:48 AM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/13/2024 3:48 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

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MERCY LAB CV - 01/13/2024 3:48 AM PST
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Narrative
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

NON-GYN CYTOPATHOLOGY - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Case Report	Cytopathology Report - Non-gyn Case: CC24-00016 Authorizing Provider: Jones, Christopher W, MD Collected: 01/13/2024 0202 Ordering Location: Scripps Mercy Hospital Received: 01/15/2024 1031 Chula Vista ICU Pathologist: Zhang, Sha, MD Specimen: Pleural cavity, right, body fluid			01/16/2024 2:12 PM PST	MERCY LAB CV	
Diagnosis	A: Pleural cavity, right, body fluid, cytology: Satisfactory for evaluation Negative for malignancy Acute inflammation and histiocytes			01/16/2024 2:12 PM PST	MERCY LAB CV	Electronically signed by Zhang, Sha, MD, PhD on 1/16/2024 at 1412 PST
Microscopic Examination	The microscopic findings support the above interpretation.			01/16/2024 2:12 PM PST	TORREY PINES LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Gross Examination	A Specimen received: volume: 2.5 cc, color: cloudy beige with debris. Specimen is forwarded to Green cytology for thinprep, cytospin, and cell block preparation. Mm TIF: 13:13 Slides prepared: 1 ThinPrep Pap stained monolayer, 1 Diff-Quik stained cytospin slide and 1 H&E stained cell block.			01/16/2024 2:12 PM PST	TORREY PINES LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Disclaimer	Processing and primary screening were performed at Scripps Medical Laboratories, Torrey Pines, 10666 N. Torrey Pines Road, La Jolla, CA 92037. Histology technical services were performed at Scripps Anatomic Pathology Laboratory, 9834 Genesee Avenue, La Jolla, CA 92037 Microscopic examination was performed at San Diego Pathologists Medical Group, 435 H Street, Chula Vista, CA 91910. If immunohistochemical staining test(s) were performed on this case at Scripps Anatomic Pathology Laboratory, they were developed and their performance characteristics determined by Scripps Anatomic Pathology Laboratory (9834 Genesee Avenue, La Jolla, CA 92037). It has not been cleared or approved by the U.S. Food and Drug Administration. The FDA has determined that such approval is not necessary. This test is used for clinical purposes. It should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA) as qualified to perform high complexity clinical laboratory testing. All controls show appropriate reactivity for each antibody reported.			01/16/2024 2:12 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Clinical History	Dx: Oral thrush Pleural effusion Prolonged QT interval COPD exacerbation (CMS/HCC) Acute kidney injury (CMS/HCC) Acute respiratory failure with hypercapnia (CMS/HCC) 59 yo F h/o HFrEF, Asthma, presents with shortness of breath, found to have concern for empyema			01/16/2024 2:12 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/15/2024 10:31 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB PATHOLOGY/ CYTOLOGY ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284
TORREY PINES LAB	10666 North Torrey Pines Rd	La Jolla, CA 92037	858-554-9560

SURVEILLANCE MRSA CULTURE - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Staph Screen culture	NO Methicillin/ Oxacillin Resistant Staphylococcus aureus		MIC	01/14/2024 6:28 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasal structure / Unknown		01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

FUNGUS CULTURE - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Result	No Fungus Isolated Final		MIC	02/02/2024 7:51 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

AFB CULTURE W/STAIN - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
AFB Culture	No AFB isolated Final		MIC	03/09/2024 6:07 PM PST	SCRIPPS LAB MICROBIOLOGY	
AFB Smear	No AFB seen by fluorescent stain			03/09/2024 6:07 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

CHOLESTEROL,BODY FLUID (CHOLESTEROL, BODY FLUID) - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cholesterol Fluid Source	Pleural fluid			01/15/2024 11:53 AM PST	ARUP LAB	
Cholesterol (FL)	38	mg/dL		01/15/2024 11:53 AM PST	ARUP LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

```
INTERPRETIVE INFORMATION: Cholesterol, Body Fluid

For information on body fluid reference ranges and/or interpretive
guidance visit http://aruplab.com/bodyfluids/

This test was developed and its performance characteristics
determined by ARUP Laboratories. It has not been cleared or
approved by the US Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for
clinical purposes.
Performed By: ARUP Laboratories
500 Chipeta Way
Salt Lake City, UT 84108
Laboratory Director: Jonathan R. Genzen, MD, PhD
CLIA Number: 46D0523979
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Pleural Fluid			01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ARUP LAB	500 Chipeta Way	Salt Lake City, UT 84108-1221	800-522-2787

TRIGLYCERIDES BODY FLUID (TRIGLYCERIDES, BODY FLUIDS) - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Triglycerides Fluid Source	Pleural fluid			01/15/2024 11:53 AM PST	ARUP LAB	
Triglycerides (FL)	38	mg/dL		01/15/2024 11:53 AM PST	ARUP LAB	

Comment:

```
INTERPRETIVE INFORMATION: Triglycerides, Fluid

For information on body fluid reference ranges and/or interpretive
guidance visit http://aruplab.com/bodyfluids/

This test was developed and its performance characteristics
determined by ARUP Laboratories. It has not been cleared or
approved by the US Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for
clinical purposes.
Performed By: ARUP Laboratories
500 Chipeta Way
Salt Lake City, UT 84108
Laboratory Director: Jonathan R. Genzen, MD, PhD
CLIA Number: 46D0523979
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Pleural Fluid			01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ARUP LAB	500 Chipeta Way	Salt Lake City, UT 84108-1221	800-522-2787

12-LEAD ECG - Final result (01/13/2024 12:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	118	bpm			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	92	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	294	ms			SCRIPPS RADIOLOGY	
QTC	412	ms			SCRIPPS RADIOLOGY	
R AXIS	60	degrees			SCRIPPS RADIOLOGY	
T AXIS	225	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 12:14 AM PST	01/15/2024 8:57 AM PST

Impressions

SCRIPPS RADIOLOGY - 01/15/2024 8:57 AM PST
Atrial fibrillation with rapid ventricular response
Left ventricular hypertrophy with repolarization abnormality (Cornell product)

Narrative

This result has an attachment that is not available.

Procedure Note

Arvin Narula, MD - 01/15/2024
Formatting of this note might be different from the original.
IMPRESSION:
Atrial fibrillation with rapid ventricular response
Left ventricular hypertrophy with repolarization abnormality (Cornell product)

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

GLUCOSE, POC - Final result (01/13/2024 12:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	121	70 - 125 mg/dL		01/13/2024 12:05 AM PST	MERCY LAB CV	
Operator ID	157,150			01/13/2024 12:05 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 12:01 AM PST	01/13/2024 12:05 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/12/2024 9:48 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/17/2024 11:01 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of right hand / Unknown	Venipuncture / Unknown	01/12/2024 9:48 PM PST	01/12/2024 10:02 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/12/2024 9:30 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/17/2024 11:01 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Non-Lab Collection / Unknown	01/12/2024 9:30 PM PST	01/12/2024 10:03 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS MICROSCOPIC W/ REFLEX TO CULTURE - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC (UA)	10-20	0 - 2 / HPF		01/12/2024 9:43 PM PST	MERCY LAB CV	
RBC (UA)	6-10	0 - 2 / HPF		01/12/2024 9:43 PM PST	MERCY LAB CV	
Epithelial Cells, Squamous (UA)	Few	None / LPF		01/12/2024 9:43 PM PST	MERCY LAB CV	
Bacteria (UA)	Few	None / HPF		01/12/2024 9:43 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

FENTANYL, URINE - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fentanyl, Interpretation (Ur)	Negative	Negative		01/12/2024 9:43 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS, MACROSCOPIC, W/REFLEX TO MICROSCOPIC, W/REFLEX TO CULTURE (URINALYSIS, CULTURE IF INDICATED) - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Color (UA)	Yellow	Yellow		01/12/2024 9:37 PM PST	MERCY LAB CV	
Clarity (UA)	Slightly Cloudy	Clear		01/12/2024 9:37 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Specific Gravity (UA)	1.020	1.005 - 1.030		01/12/2024 9:37 PM PST	MERCY LAB CV	
pH (UA)	5.5	5.0 - 8.5 pH		01/12/2024 9:37 PM PST	MERCY LAB CV	
Protein (UA)	30	Negative mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	
Glucose (UA)	Negative	Negative mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	
Ketones (UA)	Negative	Negative mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	
Bilirubin (UA)	Negative	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Occult Blood (UA)	Moderate	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Leukocyte Esterase (UA)	Small	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Nitrite (UA)	Positive	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Urobilinogen (UA)	0.2	<2.0 mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RAPID DRUG SCREEN, URINE (DRUG SCREEN, URINE) - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Benzodiazepines (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Cocaine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Tetrahydrocannabinol (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 50						

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Opiates (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						
Methamphetamine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Barbiturates (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Amphetamine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Methadone (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Phencyclidine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 25						
Tricyclic Antidepressants (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 300						
Oxycodone (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

MERCY LAB CV - 01/12/2024 9:48 PM PST
This method provides screening results for medical purposes only. A more specific alternate chemical method must be used for a confirmed analytical result. Clinical considerations and professional judgement should be applied to any drug of abuse result, particularly when screening positive results are used.

Contact laboratory to order confirmatory testing.

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINE CULTURE - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Urine Culture	Multiple organisms present, probable skin/ urogenital contamination.		MIC	01/15/2024 8:35 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:35 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/12/2024 8:49 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/12/2024 8:52 PM PST	

Impressions

01/12/2024 8:53 PM PST
IMPRESSION:
Large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly.

release

Narrative

01/12/2024 8:53 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/12/2024 8:49 PM

HISTORY: sepsis

COMPARISON: 12/22/2021

FINDINGS:
One view obtained.

Large right pleural effusion with subjacent atelectasis of the right lung. Left lung appears clear but appears hyperinflated. No pneumothorax.

The cardiomediastinal silhouette is enlarged. The osseous structures demonstrate no acute abnormality.

Procedure Note

Anshuman Bansal, MD - 01/12/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/12/2024 8:49 PM

HISTORY: sepsis

COMPARISON: 12/22/2021

FINDINGS:
One view obtained.

Large right pleural effusion with subjacent atelectasis of the right lung. Left lung appears clear but appears hyperinflated. No pneumothorax.

The cardiomediastinal silhouette is enlarged. The osseous structures demonstrate no acute abnormality.

IMPRESSION:
IMPRESSION:
Large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly.

release

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

BLOOD GAS LACTIC ACID, VENOUS - Final result (01/12/2024 7:44 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Venous	7.26	7.35 - 7.45		01/12/2024 7:53 PM PST	MERCY LAB CV	
pCO2, Venous	66	38 - 50 mm Hg		01/12/2024 7:53 PM PST	MERCY LAB CV	
pO2, Venous	28	Not Established mm Hg		01/12/2024 7:53 PM PST	MERCY LAB CV	
HCO3, Venous	29.5	22.0 - 29.0 mmol/L		01/12/2024 7:53 PM PST	MERCY LAB CV	
O2 Sat, Venous	41.8	60.0 - 85.0 %		01/12/2024 7:53 PM PST	MERCY LAB CV	
Base Excess/Deficit, Venous	2.3	Not Established mmol/L		01/12/2024 7:53 PM PST	MERCY LAB CV	
Lactic Acid, Venous	2.9	0.9 - 2.5 mmol/L		01/12/2024 7:53 PM PST	MERCY LAB CV	
Notification Note	CRITICAL VPH/VPCO2			01/12/2024 7:53 PM PST	MERCY LAB CV	
Notified By	128296			01/12/2024 7:53 PM PST	MERCY LAB CV	
Notified Whom	EMMA 241391 ER RN			01/12/2024 7:53 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Operator ID	128,296			01/12/2024 7:53 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:44 PM PST	01/12/2024 7:46 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COVID-19, RAPID COMBO (COVID-19, RAPID (PCR/NAAT, COMBO)) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected		01/12/2024 8:53 PM PST	MERCY LAB CV	

Comment:

A Detected result is considered a positive result for that virus.
A positive SARS-CoV-2 result indicates that RNA from SARS-CoV-2 was detected and that the patient is considered infected and presumed contagious. A positive Influenza A or B result indicates that RNA from Influenza A or B, respectively, was detected and that the patient is considered infected and presumed contagious.

A Not Detected (negative) test result means that RNA was not present in the specimen above the limit of detection. However, it does not rule out the possibility of infection and should not be used as the sole basis for patient management decisions. Test parameters for asymptomatic patients have not been evaluated.

These results have been reported to the San Diego Dept. of Health Services as required by Title 17, California Code of regulations, Section 2500.
Test information for patients can be found at the following link: https://www.fda.gov/media/142192/download
Test information for providers can be found at the following link: https://www.fda.gov/media/142191/download

This SARS-CoV-2 Real Time RT-PCR test was performed under an Emergency Use Authorization granted by the FDA for the Roche Liat.

This test meets Covid-19 testing requirements for school and business testing criteria and is not an antibody or antigen test.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

INFLUENZA A/B COMBO - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Influenza A PCR	Not Detected	Not Detected		01/12/2024 8:53 PM PST	MERCY LAB CV	
Influenza B PCR	Not Detected	Not Detected		01/12/2024 8:53 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

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MERCY LAB CV - 01/12/2024 8:53 PM PST
These results have been reported to the San Diego Department of Health Services as required by Title 17,
California Code of Regulations, Section 2505.
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Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

THYROID STIMULATING HORMONE (ED) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Thyroid Stimulating Hormone	1.260	0.465 - 4.680 mcIU/mL		01/12/2024 8:31 PM PST	MERCY LAB CV	

Comment: High dose biotin supplementation (>1 mg/day) may falsely decrease results.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	13,700	Refer to the reference range comment. pg/mL		01/12/2024 8:12 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

NT-proBNP reference range: <=125 pg/mL

NT-proBNP testing aids in the diagnosis or exclusion of heart failure (HF) when used in conjunction with clinical history, symptoms, exam findings, and additional imaging and laboratory studies.

Emergency Department:
The ICON-RELOADED study provided validation of cutoff values for NT-proBNP in patients presenting with dyspnea in the Emergency Department (1).
In this setting NT-proBNP level <300 pg/mL has a high negative predictive value for HF. For values above 300 pg/mL, age-specific cutoffs provide additional decision limit categories:

Result.....Age.....Interpretation
<300.....All.....Negative: Heart Failure Unlikely
=300 to <450.....22-<50..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <900.....50-<75..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <1800....>=75....Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=>450.....22-<50..Positive: Heart Failure Likely
=>900.....50-<75..Positive: Heart Failure Likely
=>1800.....>=75....Positive: Heart Failure Likely

* Natriuretic peptides values in the gray zones may be caused by conditions other than heart failure. Acute coronary syndrome,pulmonary embolism,pulmonary hypertension,sepsis,stroke,and renal dysfunction will elevate NT- proBNP levels. Obesity, flash pulmonary edema,cardiac tamponade,and pericardial constriction are conditions associated with reduced NT-proBNP(2).

Ambulatory Clinics:
For patients presenting to outpatient clinics with suspicion of HF, the optimal clinical utility of NT-proBNP is to
exclude heart failure.For this clinical setting Ortho Clinical Diagnostics recommends NT-proBNP results be interpreted
as in the table below(2):

Result.....Age.....Interpretation
<125.....All.....Negative: Heart Failure Unlikely
>=125.....All.....Consider Heart Failure as well as other causes* of NT-proBNP elevation.

* Natriuretic peptides value elevations could also be caused by conditions other than heart failure such as acute
coronary syndrome, pulmonary embolism, pulmonary hypertension, sepsis, stroke, and renal dysfunction (2).

References:
1. Terminal Pro-B-Type Natriuretic Peptide in the Emergency Department: The ICON-RELOADED Study. J Am Coll Cardiol. 2018 Mar 20;71(11):1191-1200. doi: 10.1016/j.jacc.2018.01.021. PMID: 29544601.

2. NBNP2 Instructions For Use. Ortho Clinical Diagnostics

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROCALCITONIN - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Procalcitonin	0.29	0.03 - 0.08 ng/mL		01/13/2024 2:38 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

MERCY LAB CV - 01/13/2024 2:38 AM PST
Procalcitonin (PCT) is a dynamic biomarker with value in guiding antibiotic management in select clinical settings. PCT values are most useful when trends are analyzed. Decisions on antibiotic use should not be based solely on PCT level. Consider repeating PCT no more than daily in ICU/sepsis patients, and every 2 days for LRTI in order to discontinue antibiotics.

Suspected Lower Respiratory Tract Infection (LRTI):
0.1-0.25 ng/mL - low likelihood for bacterial infection, antibiotics discouraged
>0.25 ng/mL - increased likelihood for bacterial infection.

Suspected Sepsis
0.1-0.5 ng/mL - low likelihood for sepsis, antibiotics discouraged.
>0.5 ng/mL - increased likelihood of sepsis.
>2.0 ng/mL - high risk of sepsis/septic shock.

Consider discontinuation of antibiotics when PCT <0.25 (LRTI) or <0.5 (sepsis), or 80% reduction from baseline.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	18.7	3.4 - 11.0 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
RBC	4.40	3.98 - 5.25 M/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
HGB	13.2	11.9 - 15.3 g/dL		01/12/2024 9:40 PM PST	MERCY LAB CV	
HCT	43.7	37.3 - 46.7 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
MCV	99	81 - 100 fL		01/12/2024 9:40 PM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		01/12/2024 9:40 PM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		01/12/2024 9:40 PM PST	MERCY LAB CV	
RDW	14.8	11.7 - 14.9 %		01/12/2024 9:40 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	799	150 - 425 K/ mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
MPV	9.2	9.0 - 12.8 fL		01/12/2024 9:40 PM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/12/2024 9:40 PM PST	MERCY LAB CV	
Preliminary Abs Neut	16.76	1.50 - 7.40 K/ mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.029	<=0.034 ng/mL		01/12/2024 8:12 PM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population. This patient's result is considered normal. However, clinical
correlation is suggested to determine whether serial measurements are necessary for further evaluation.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MANUAL DIFFERENTIAL,REFLEXED FROM AUTO (MANUAL DIFFERENTIAL,REFLEXED) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Neutrophils %	93.0	38.0 - 74.0 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Bands %	3	0 - 6 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Lymphocytes %	2.0	16.0 - 48.0 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Monocytes %	2.0	4.9 - 12.5 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Eosinophils %	0.0	0.0 - 9.5 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Basophils %	0.0	0.0 - 1.6 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Neutrophils	17.39	1.50 - 7.40 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Bands	0.56	<=0.60 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Lymphocytes	0.37	0.90 - 3.10 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Monocytes	0.37	0.26 - 0.87 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Eosinophils	0.00	0.00 - 0.51 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Basophils	0.00	0.00 - 0.09 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
PLT Slide Review	Verified by slide scan.			01/12/2024 9:40 PM PST	MERCY LAB CV	
RBC Slide Review	Reviewed			01/12/2024 9:40 PM PST	MERCY LAB CV	
Ovalocytes	Few			01/12/2024 9:40 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

APTT (ACTIVATED PTT) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
aPTT	31	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds		01/12/2024 8:04 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTIME-INR (PROTHROMBIN TIME-INR) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protime	13.3	10.0 - 13.1 Seconds		01/12/2024 8:04 PM PST	MERCY LAB CV	
INR	1.1	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation		01/12/2024 8:04 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LIPASE - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Lipase	114	23 - 300 Units/L		01/13/2024 2:15 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/12/2024 8:01 PM PST	MERCY LAB CV	
Glucose	132	70 - 125 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Sodium	137	137 - 145 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Potassium	5.1	3.5 - 5.1 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Urea Nitrogen	48	7 - 17 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Creatinine	1.09	0.50 - 1.00 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	59	>60 mL/min/1.73m*2		01/12/2024 8:01 PM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Osmolality Calculated	298	280 - 305 mOsm/kg H20		01/12/2024 8:01 PM PST	MERCY LAB CV	
Calcium	9.6	8.4 - 10.2 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Protein, total	8.8	6.3 - 8.2 g/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Albumin	3.7	3.5 - 5.0 g/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Alkaline Phosphatase	133	38 - 126 Units/L		01/12/2024 8:01 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
AST	70	14 - 36 Units/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
ALT	31	<=34 Units/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Bilirubin, Total	0.8	0.2 - 1.3 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

MERCY LAB CV - 01/12/2024 8:01 PM PST

```
for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (01/12/2024 7:36 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	152	bpm			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	98	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	318	ms			SCRIPPS RADIOLOGY	
QTC	505	ms			SCRIPPS RADIOLOGY	
R AXIS	69	degrees			SCRIPPS RADIOLOGY	
T AXIS	212	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/12/2024 7:36 PM PST	01/15/2024 1:15 PM PST

Impressions

SCRIPPS RADIOLOGY - 01/15/2024 1:15 PM PST

Impressions

```
** Critical Test Result: High HR
Atrial fibrillation with rapid ventricular response
Minimal voltage criteria for LVH, may be normal variant ( Cornell product )
Septal infarct , age undetermined
Abnormal ECG
When compared with ECG of 22-Dec-2021 20:06,
Atrial fibrillation has replaced Sinus rhythm
Septal infarct is now present
Nonspecific T wave abnormality, worse in Inferior leads
```

Narrative

This result has an attachment that is not available.

Procedure Note

Stephen Zanoni, MD - 01/15/2024

Formatting of this note might be different from the original.

IMPRESSION:
** Critical Test Result: High HR
Atrial fibrillation with rapid ventricular response
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Septal infarct , age undetermined
Abnormal ECG
When compared with ECG of 22-Dec-2021 20:06,
Atrial fibrillation has replaced Sinus rhythm
Septal infarct is now present
Nonspecific T wave abnormality, worse in Inferior leads

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

BLOOD GAS, ARTERIAL - Final result (12/23/2021 1:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.44	7.35 - 7.45		12/23/2021 1:47 PM PST	MERCY LAB CV	
pCO2, Arterial	59	35 - 45 mm Hg		12/23/2021 1:47 PM PST	MERCY LAB CV	
pO2, Arterial	81	83 - 108 mm Hg		12/23/2021 1:47 PM PST	MERCY LAB CV	
HCO3, Arterial	40.0	21.0 - 28.0 mmol/L		12/23/2021 1:47 PM PST	MERCY LAB CV	
O2 Sat, Arterial	96.7	95.0 - 98.0 %		12/23/2021 1:47 PM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	15.9	-2.0 - 3.0 mmol/L		12/23/2021 1:47 PM PST	MERCY LAB CV	
Oxygen Device	Room Air			12/23/2021 1:47 PM PST	MERCY LAB CV	
Modes	NA			12/23/2021 1:47 PM PST	MERCY LAB CV	
Notified By	243446			12/23/2021 1:47 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Notified Whom	166427			12/23/2021 1:47 PM PST	MERCY LAB CV	
Operator ID	243,446			12/23/2021 1:47 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	12/23/2021 1:41 PM PST	12/23/2021 1:47 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
John-Vincent Arcilla MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (12/23/2021 5:20 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	8.0	3.4 - 11.0 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
RBC	3.82	3.98 - 5.25 M/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
HGB	11.9	11.9 - 15.3 g/dL		12/23/2021 8:07 AM PST	MERCY LAB CV	
HCT	38.0	37.3 - 46.7 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		12/23/2021 8:07 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		12/23/2021 8:07 AM PST	MERCY LAB CV	
MCHC	31.3	31.0 - 36.0 g/dL		12/23/2021 8:07 AM PST	MERCY LAB CV	
RDW	13.8	11.7 - 14.9 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Platelet Count	342	150 - 425 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
nRBC	0.0	0 /100 WBCs		12/23/2021 8:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	5.80	1.50 - 7.40 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Neutrophils %	72.4	38.0 - 74.0 %		12/23/2021 8:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
IG%	0.4	0.0 - 1.2 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Lymphocytes %	20.9	16.0 - 48.0 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Monocytes %	4.0	4.9 - 12.5 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Eosinophils %	1.8	0.0 - 9.5 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Basophils %	0.5	0.0 - 1.6 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	5.80	1.50 - 7.40 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.67	0.90 - 3.10 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.32	0.26 - 0.87 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.14	0.00 - 0.51 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/ mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/23/2021 5:20 AM PST	12/23/2021 7:54 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
John-Vincent Arcilla MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (12/23/2021 5:20 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/23/2021 6:13 AM PST	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	97	98 - 107 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
Glucose	109	70 - 125 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Urea Nitrogen	31	7 - 17 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Creatinine	0.9	0.5 - 1.0 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Osmolality Calculated	293	280 - 305 mOsm/ kg H20		12/23/2021 6:13 AM PST	MERCY LAB CV	
Albumin	3.1	3.5 - 5.0 g/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Protein, total	6.1	6.3 - 8.2 g/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Alkaline Phosphatase	83	38 - 126 Units/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
AST	38	14 - 36 Units/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
Bilirubin, Total	0.4	0.2 - 1.3 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
ALT	32	<=34 Units/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
GFR Calculation, African American	>60	>60 mL/ min/ 1.73m*2		12/23/2021 6:13 AM PST	MERCY LAB CV	
GFR Calculation, Non-African American	>60	>60 mL/ min/ 1.73m*2		12/23/2021 6:13 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/23/2021 5:20 AM PST	12/23/2021 5:42 AM PST

Narrative

MERCY LAB CV - 12/23/2021 6:13 AM PST
eGFR not calculated on patients <18 years of age.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RAPID DRUG SCREEN, URINE (DRUG SCREEN, URINE) - Final result (12/22/2021 10:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Benzodiazepines (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Cocaine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Tetrahydrocannabinol (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 50						
Opiates (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						
Methamphetamine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Barbiturates (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Amphetamine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Methadone (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Phencyclidine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 25						
Tricyclic Antidepressants (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 300						
Oxycodone (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen collection, clean catch / Unknown		12/22/2021 10:41 PM PST	12/22/2021 10:43 PM PST

Narrative

MERCY LAB CV - 12/22/2021 11:04 PM PST
This method provides screening results for medical purposes only. A more specific alternate chemical method must be used for a confirmed analytical result. Clinical considerations and professional judgement should be applied to any drug of abuse result, particularly when screening positive results are used.

Contact laboratory to order confirmatory testing.

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS DIPSTICK W/ REFLEX TO MICROSCOPIC (URINALYSIS WITH MICROSCOPIC IF INDICATED) - Final result (12/22/2021 10:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Color (UA)	Yellow	Yellow		12/22/2021 10:46 PM PST	MERCY LAB CV	
Clarity (UA)	Clear	Clear		12/22/2021 10:46 PM PST	MERCY LAB CV	
Specific Gravity (UA)	1.010	1.005 - 1.030		12/22/2021 10:46 PM PST	MERCY LAB CV	
pH (UA)	6.0	5.0 - 8.5 pH		12/22/2021 10:46 PM PST	MERCY LAB CV	
Protein (UA)	Negative	Negative mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	
Glucose (UA)	Negative	Negative mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	
Ketones (UA)	Negative	Negative mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	
Bilirubin (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Occult Blood (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Leukocyte Esterase (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Nitrite (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Urobilinogen (UA)	0.2	<2.0 mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen collection, clean catch / Unknown		12/22/2021 10:41 PM PST	12/22/2021 10:43 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COVID-19, RAPID COMBO (COVID-19, RAPID (PCR/NAAT, COMBO)) - Final result (12/22/2021 9:08 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected		12/22/2021 9:37 PM PST	MERCY LAB CV	

Comment:

A Detected result is considered a positive result for that virus.
A positive SARS-CoV-2 result indicates that RNA from SARS-CoV-2 was detected and that the patient is considered infected and presumed contagious. A positive Influenza A or B result indicates that RNA from Influenza A or B, respectively, was detected and that the patient is considered infected and presumed contagious.

A Not Detected (negative) test result means that RNA was not present in the specimen above the limit of detection. However, it does not rule out the possibility of infection and should not be used as the sole basis for patient management decisions. Test parameters for asymptomatic patients have not been evaluated.

These results have been reported to the San Diego Dept. of Health Services as required by Title 17, California Code of regulations, Section 2500.
Test information for patients can be found at the following link: https://www.fda.gov/media/142192/download
Test information for providers can be found at the following link: https://www.fda.gov/media/142191/download

This SARS-CoV-2 Real Time RT-PCR test was performed under an Emergency Use Authorization granted by the FDA for the Roche Liat.

This test meets Covid-19 testing requirements for school and business testing criteria and is not an antibody or antigen test.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		12/22/2021 9:08 PM PST	12/22/2021 9:11 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

INFLUENZA A/B COMBO - Final result (12/22/2021 9:08 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Influenza A PCR	Not Detected	Not Detected		12/22/2021 9:37 PM PST	MERCY LAB CV	
Influenza B PCR	Not Detected	Not Detected		12/22/2021 9:37 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		12/22/2021 9:08 PM PST	12/22/2021 9:11 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (12/22/2021 8:29 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/22/2021 8:37 PM PST	

Impressions

12/22/2021 8:39 PM PST
IMPRESSION:

Left midlung and retrocardiac opacities likely reflect atelectasis, although a developing infection cannot be completely excluded. Right lung is clear. No pulmonary edema.

Moderate cardiomegaly.

Small left pleural effusion. No pneumothorax.

Narrative

12/22/2021 8:39 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/22/2021 8:29 PM

HISTORY: chest pain

COMPARISON: None.

FINDINGS:

See impression.

Procedure Note

Christopher Maxwell, MD - 12/22/2021

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/22/2021 8:29 PM

HISTORY: chest pain

COMPARISON: None.

FINDINGS:

See impression.

IMPRESSION:
IMPRESSION:

Left midlung and retrocardiac opacities likely reflect atelectasis, although a developing infection cannot be completely excluded. Right lung is clear. No pulmonary edema.

Moderate cardiomegaly.

Small left pleural effusion. No pneumothorax.

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	9,890.0	<=125.0 pg/mL		12/22/2021 9:19 PM PST	MERCY LAB CV	

Comment:

```
Optimal Cut-Points (pg/mL)
<50 years old:  <= 450
50-75 years old: <= 900
>75 years old: <= 1800
Reference: Januzzi JL et. al.
European Heart Journal. 2006, 27: 330-337

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	9.2	3.4 - 11.0 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
RBC	3.91	3.98 - 5.25 M/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
HGB	12.1	11.9 - 15.3 g/dL		12/22/2021 8:54 PM PST	MERCY LAB CV	
HCT	39.3	37.3 - 46.7 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		12/22/2021 8:54 PM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		12/22/2021 8:54 PM PST	MERCY LAB CV	
MCHC	30.8	31.0 - 36.0 g/dL		12/22/2021 8:54 PM PST	MERCY LAB CV	
RDW	13.5	11.7 - 14.9 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Platelet Count	348	150 - 425 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
nRBC	0.0	0 /100 WBCs		12/22/2021 8:54 PM PST	MERCY LAB CV	
Preliminary Abs Neut	6.74	1.50 - 7.40 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Neutrophils %	73.7	38.0 - 74.0 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Lymphocytes %	19.3	16.0 - 48.0 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Monocytes %	4.2	4.9 - 12.5 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Eosinophils %	1.5	0.0 - 9.5 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Basophils %	0.5	0.0 - 1.6 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Neutrophils	6.74	1.50 - 7.40 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
IG#	0.07	0.00 - 0.10 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	1.77	0.90 - 3.10 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Monocytes	0.38	0.26 - 0.87 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Eosinophils	0.14	0.00 - 0.51 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.035	<=0.034 ng/mL		12/22/2021 9:19 PM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population.  This patient's result is suspicious for myocardial injury. However,
serial measurements may be necessary to confirm or exclude the diagnosis of acute myocardial infarction.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TSH (THYROID STIMULATING HORMONE) - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Thyroid Stimulating Hormone	0.679	0.465 - 4.680 mcIU/mL		12/23/2021 2:20 AM PST	MERCY LAB CV	

Comment: High dose biotin supplementation (>1 mg/day) may falsely decrease results.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LIPID PANEL (LIPID PANEL WITH CALC LDL) - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/23/2021 10:28 AM PST	SCRIPPS LAB	
Triglycerides	59	<150 mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	

Comment:

```
Normal          <150     mg/dL
Borderline High  150-199 mg/dL
High             200-499 mg/dL
Very high       >499     mg/dL
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cholesterol	122	<200 mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	

Comment:

```
Desirable        <200     mg/dL
Borderline High   200-239 mg/dL
High             >239     mg/dL
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HDL Cholesterol	62	Not Established mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	

Comment:

```
Low  <40 mg/dL
High >59 mg/dL(Negative Risk Factor)
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
LDL Calculation	48	<130 mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

SCRIPPS LAB - 12/23/2021 10:28 AM PST

Narrative

Reference ranges are derived from the clinical practice ATPIII Guidelines of the National Cholesterol Education Program (NCEP) for the prevention and management of high cholesterol in adults (September 2002).

The relationship between levels of LDL cholesterol and Coronary Heart Disease is continuous over a broad range of LDL levels. A reduction in Cholesterol, Triglycerides and LDL levels has been associated with a reduced risk of CHD.

http://www.nhlbi.nih.gov/guidelines/cholesterol/index.htm

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

BASIC METABOLIC PANEL - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/22/2021 9:07 PM PST	MERCY LAB CV	
Sodium	139	137 - 145 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Potassium	4.1	3.5 - 5.1 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
CO2, Total	35	22 - 30 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Urea Nitrogen	28	7 - 17 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
Creatinine	0.7	0.5 - 1.0 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
Glucose	109	70 - 125 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
Osmolality Calculated	294	280 - 305 mOsm/kg H20		12/22/2021 9:07 PM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
GFR Calculation, Non-African American	>60	>60 mL/min/1.73m*2		12/22/2021 9:07 PM PST	MERCY LAB CV	
GFR Calculation, African American	>60	>60 mL/min/1.73m*2		12/22/2021 9:07 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

MERCY LAB CV - 12/22/2021 9:07 PM PST
eGFR not calculated on patients <18 years of age.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (12/22/2021 8:06 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	113	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	113	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	146	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	84	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	340	ms			SCRIPPS RADIOLOGY	
QTC	466	ms			SCRIPPS RADIOLOGY	
P AXIS	62	degrees			SCRIPPS RADIOLOGY	
R AXIS	40	degrees			SCRIPPS RADIOLOGY	
T AXIS	90	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/22/2021 8:06 PM PST	12/23/2021 7:27 AM PST

Impressions

SCRIPPS RADIOLOGY - 12/23/2021 7:28 AM PST
Sinus tachycardia
Possible Left atrial enlargement
Left ventricular hypertrophy with repolarization abnormality (Cornell product)
Abnormal ECG
No previous ECGs available

Narrative

This result has an attachment that is not available.

Procedure Note

Douglas Ballesteros, MD - 12/23/2021
Formatting of this note might be different from the original.
IMPRESSION:
Sinus tachycardia
Possible Left atrial enlargement
Left ventricular hypertrophy with repolarization abnormality (Cornell product)
Abnormal ECG
No previous ECGs available

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

Visit Diagnoses - as of 10/13/2026

Diagnosis	Start Date
Heart failure	12/22/2021
Chest pain, unspecified type	12/22/2021
Respiratory insufficiency	1/12/2024
COPD exacerbation	1/12/2024
Fluid in pleural cavity	1/12/2024
Oral yeast infection	1/12/2024
Acute kidney injury	1/12/2024
Prolonged Q-T interval on ECG	1/12/2024
Loculated pleural effusion	1/12/2024
Heart failure	1/12/2024
Heart failure	8/14/2024
Shortness of breath	8/14/2024
History of noncompliance with medical treatment	8/14/2024
History of pleural empyema	8/14/2024
Heart failure	9/23/2024
Hypotension, unspecified hypotension type	12/24/2024
High blood pressure	12/26/2024
Leg hematoma, left, initial encounter	6/22/2025
Left leg pain	6/22/2025
Heart failure	12/22/2021
Fluid in the lungs	12/22/2021
Asthma	12/22/2021
High blood pressure	12/22/2021
Pus collection in space between lung and chest wall	1/12/2024

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers	Document Coverage Dates
	Jul. 29, 1964 - Oct. 13, 2026

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

Elizabeth Tina Petruccio

Summary of Care, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Reason for Visit

Reason
Leg Swelling

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
06/22/2025 10:22 PM PDT - 06/23/2025 3:28 AM PDT	Emergency	Scripps Mercy Hospital Chula Vista Emergency Department 435 H Street Chula Vista, CA 91910-4307 619-691-7000	**Miguel Mendoza Navarro, MD** 435 H St Chula Vista, CA 91910-4307 619-691-7290 (Work) 619-691-7435 (Fax)	Discharge Disposition: Disch/ Trans Other HC Inst

Allergies - documented as of this encounter (statuses as of 10/13/2026)

Active Allergy	Reactions	Criticality	Noted Date	Comments
Clindamycin	Other (see comments)		09/23/2024	Patient unsure of what reaction she has from medication.
Lactose	Other (see comments)	Low	01/20/2024	Patient having diarrhea; only fluid milk - all other lactose items ok

Inactive Allergy	Reactions	Criticality	Noted Date	Inactivity Date	Comments
Lincosamides	Other (see comments), Unknown Reaction		07/11/2012	09/23/2024	

Medications - documented as of this encounter (statuses as of 10/13/2026)

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.						Active
ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.				02/16/2024		Active
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			02/17/2024		Active
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3		08/14/2024		Active
gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule			09/18/2024		Active
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule			09/23/2024		Active
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024		Active
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/ actuation inhaler	INHALE 2 PUFFS EVERY 4 (FOUR) HOURS AS NEEDED FOR WHEEZING. CALOPTIMA MCAL ID 93931022D30310	90 g	3		10/15/2024		Active
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3		08/14/2024	11/02/2025	Discontinued
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3		08/14/2024	11/02/2025	Discontinued
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3		10/17/2024	11/03/2025	Discontinued
metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	TAKE 1 TABLET (25 MG TOTAL) BY MOUTH DAILY.	90 tablet			04/10/2025	11/04/2025	Discontinued

Active Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date
Gallstones	02/18/2024	
Gall bladder pain	02/18/2024	
Pus collection in space between lung and chest wall	01/26/2024	
Acute osteomyelitis of cervical spine	01/26/2024	
Heart failure	01/13/2024	
Loculated pleural effusion	01/13/2024	
Atrial fibrillation (irregular heartbeat)	01/13/2024	
Respiratory insufficiency	01/12/2024	
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Resolved Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date	Resolved Date
Severe sepsis	02/18/2024		03/24/2024

Immunizations - documented as of this encounter

Immunization	Administration Dates	Next Due
Tdap greater than or equal to 7yrs IM	02/09/2024	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Passive Smoke Exposure: Past				
Smokeless Tobacco: Never				

Patient History	Answer	Date Recorded
Alcohol Use Status	Not Currently	04/18/2024
Oz/Week (Alcohol Misuse Male >= 9 OR Female >= 4.8)	Not on file	04/18/2024

Housing Stability	Answer	Date Recorded
What is your living situation today?	Do not have a stable place to live	01/19/2024

Food Insecurity	Answer	Date Recorded
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024

Transportation Needs	Answer	Date Recorded
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024

Pregnant	Comments
No	

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

Last Filed Vital Signs - documented in this encounter

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	104/68	06/23/2025 3:04 AM PDT	
Pulse	89	06/23/2025 3:15 AM PDT	
Temperature	36.6 °C (97.9 °F)	06/23/2025 3:04 AM PDT	
Respiratory Rate	21	06/23/2025 3:15 AM PDT	
Oxygen Saturation	96%	06/23/2025 3:15 AM PDT	
Inhaled Oxygen Concentration	-	-	
Weight	65.9 kg (145 lb 4.5 oz)	06/22/2025 10:53 PM PDT	
Height	167.6 cm (5' 6")	06/22/2025 10:16 PM PDT	
Body Mass Index	23.45	06/22/2025 10:16 PM PDT	

Functional Status - documented as of this encounter

Prehospital Treatment

Question	Answer	Date of Assessment	Author
Prehospital Treatment	No	06/22/2025 10:13 PM PDT	Your Scripps Health Team

Tuberculosis Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	Denies symptoms	06/22/2025 10:16 PM PDT	Your Scripps Health Team
Risks Present	Homeless	06/22/2025 10:16 PM PDT	Your Scripps Health Team
Results	TB Risk Screen Negative	06/22/2025 10:16 PM PDT	Your Scripps Health Team

Oxygen Therapy

Question	Answer	Date of Assessment	Author
Patient Activity	At rest	06/22/2025 10:16 PM PDT	Your Scripps Health Team
Pulse Oximetry Type	Intermittent	06/22/2025 10:16 PM PDT	Your Scripps Health Team
Oxygen Therapy	None (Room air)	06/23/2025 3:28 AM PDT	Your Scripps Health Team

Morse Fall Risk

Question	Answer	Date of Assessment	Author
History of Falling	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Secondary Diagnosis	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Ambulatory Aids	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Intravenous Therapy/Heparin/Saline Lock	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Gait/Transferring	10	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Mental Status	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Morse Fall Risk Score	10	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Columbia Suicide Severity Rating Scale

Question	Answer	Date of Assessment	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	06/22/2025 10:17 PM PDT	Your Scripps Health Team
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	06/22/2025 10:17 PM PDT	Your Scripps Health Team
2. In the past month have you actually had any thoughts of killing yourself?	No	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Violence Risk Screening (Step 1)

Question	Answer	Date of Assessment	Author
Automatic Triggers	NA	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Risk Factor Triggers	NA	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Date of Assessment	Author
Violence Screening Outcome	Negative Violence Risk Screen	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Screening

Question	Answer	Date of Assessment	Author
Where was pt living / residing	Homeless, unsheltered	06/22/2025 10:18 PM PDT	Your Scripps Health Team

Height and Weight

Question	Answer	Date of Assessment	Author
Weight	2324.53	06/22/2025 10:53 PM PDT	Your Scripps Health Team
Weight Method	Bed Scale	06/22/2025 10:53 PM PDT	Your Scripps Health Team

Pain Assessment

Question	Answer	Date of Assessment	Author
Pain Orientation	Left;Lower	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Descriptors	Aching	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Onset	Ongoing	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Frequency	Constant/ continuous	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Patient's Stated Pain Goal	No pain	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Type	Acute pain	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Clinical Progression of Pain	Not changed	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Score	0/No SX - No pain	06/23/2025 12:49 AM PDT	Your Scripps Health Team
Pain Assessment	No/denies pain	06/23/2025 3:28 AM PDT	Your Scripps Health Team
Pain Location	Leg	06/22/2025 10:20 PM PDT	Your Scripps Health Team

Quick Updates

Question	Answer	Date of Assessment	Author
Quick Updates - Free Text	Report given to EMS for transport to UCSD Trauma	06/23/2025 3:29 AM PDT	Your Scripps Health Team

Peripheral Vascular

Question	Answer	Date of Assessment	Author
LUE Edema	Non-pitting	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Date of Assessment	Author
Skin Color	Appropriate for ethnicity	06/22/2025 10:43 PM PDT	Your Scripps Health Team
Type of Wound (LDA)	Wound	06/22/2025 10:43 PM PDT	Your Scripps Health Team
Skin Color/Condition (WDL)	X	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Endocrine

Question	Answer	Date of Assessment	Author
Endocrine (WDL)	WDL	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	06/22/2025 10:43 PM PDT	Your Scripps Health Team

BSA (Calculated - sq m)

Answer	Date of Assessment	Author
1.75	06/22/2025 10:53 PM PDT	Your Scripps Health Team

Assistive Devices at Bedside

Question	Answer	Date of Assessment	Author
Patient Assistive Devices at Bedside	None	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Assistive Devices Sent Home

Question	Answer	Date of Assessment	Author
Patient Assistive Devices Sent Home	None	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Valuables Sent to Safe/Secure Area

Question	Answer	Date of Assessment	Author
Patient Valuables Sent to Safe/Secure Area	No Valuables	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Patient Medications

Question	Answer	Date of Assessment	Author
Medications brought by patient?	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Precaution Necessity Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Chlorhexidine Daily Bath Results	No CHG bathing required	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Patient Status	None of the above	06/23/2025 12:53 AM PDT	Your Scripps Health Team
(RETIRED, READ-ONLY) History of MDRO	Greater Than 4 Months	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Patient has an Open Draining Wound that cannot be contained by dressing	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team

C.difficile Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	No Active Diarrhea	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Risks Present	None of the Above	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Results	No C. difficile test necessary	06/23/2025 12:53 AM PDT	Your Scripps Health Team

CRO and Candida auris Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
History of overnight hospitalization and/or invasive procedure outside of the United States within last 12 months	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team
History of LTAC/SNF/Subacute residency within the last 12 months	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Direct transfer or history of hospitalization in last 12mos from outside San Diego county healthcare facility (i.e. ACH, SNF, LTAC)	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Known History of CRO (Carbapenem-resistant organism)	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team
CRO Results and Interventions	Specimen not required.	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Known History of Candida auris	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Candida auris Results and Interventions	Specimen not required.	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Departure Condition

Question	Answer	Date of Assessment	Author
DC DISPO & LOCATION (RN)	Acute Care Hospital	06/23/2025 3:28 AM PDT	Your Scripps Health Team

Prehospital Treatment

Question	Answer	Date of Assessment	Author
Prehospital Treatment	No	06/22/2025 10:13 PM PDT	Your Scripps Health Team

Tuberculosis Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	Denies symptoms	06/22/2025 10:16 PM PDT	Your Scripps Health Team
Risks Present	Homeless	06/22/2025 10:16 PM PDT	Your Scripps Health Team
Results	TB Risk Screen Negative	06/22/2025 10:16 PM PDT	Your Scripps Health Team

Oxygen Therapy

Question	Answer	Date of Assessment	Author
Patient Activity	At rest	06/22/2025 10:16 PM PDT	Your Scripps Health Team
Pulse Oximetry Type	Intermittent	06/22/2025 10:16 PM PDT	Your Scripps Health Team
Oxygen Therapy	None (Room air)	06/23/2025 3:28 AM PDT	Your Scripps Health Team

Morse Fall Risk

Question	Answer	Date of Assessment	Author
History of Falling	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Secondary Diagnosis	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Ambulatory Aids	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Intravenous Therapy/Heparin/Saline Lock	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Gait/Transferring	10	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Mental Status	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Morse Fall Risk Score	10	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Columbia Suicide Severity Rating Scale

Question	Answer	Date of Assessment	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	06/22/2025 10:17 PM PDT	Your Scripps Health Team
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	06/22/2025 10:17 PM PDT	Your Scripps Health Team
2. In the past month have you actually had any thoughts of killing yourself?	No	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Violence Risk Screening (Step 1)

Question	Answer	Date of Assessment	Author
Automatic Triggers	NA	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Risk Factor Triggers	NA	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Date of Assessment	Author
Violence Screening Outcome	Negative Violence Risk Screen	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Screening

Question	Answer	Date of Assessment	Author
Where was pt living / residing	Homeless, unsheltered	06/22/2025 10:18 PM PDT	Your Scripps Health Team

Pain Assessment

Question	Answer	Date of Assessment	Author
Pain Orientation	Left;Lower	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Descriptors	Aching	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Onset	Ongoing	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Frequency	Constant/ continuous	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Patient's Stated Pain Goal	No pain	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Type	Acute pain	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Clinical Progression of Pain	Not changed	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Score	0/No SX - No pain	06/23/2025 12:49 AM PDT	Your Scripps Health Team
Pain Assessment	No/denies pain	06/23/2025 3:28 AM PDT	Your Scripps Health Team
Pain Location	Leg	06/22/2025 10:20 PM PDT	Your Scripps Health Team

Peripheral Vascular

Question	Answer	Date of Assessment	Author
LUE Edema	Non-pitting	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Date of Assessment	Author
Skin Color	Appropriate for ethnicity	06/22/2025 10:43 PM PDT	Your Scripps Health Team
Type of Wound (LDA)	Wound	06/22/2025 10:43 PM PDT	Your Scripps Health Team

HEENT

Question	Answer	Date of Assessment	Author
HEENT (WDL)	WDL	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Endocrine

Question	Answer	Date of Assessment	Author
Endocrine (WDL)	WDL	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Assistive Devices at Bedside

Question	Answer	Date of Assessment	Author
Patient Assistive Devices at Bedside	None	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Assistive Devices Sent Home

Question	Answer	Date of Assessment	Author
Patient Assistive Devices Sent Home	None	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Valuables Sent to Safe/Secure Area

Question	Answer	Date of Assessment	Author
Patient Valuables Sent to Safe/Secure Area	No Valuables	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Patient Medications

Question	Answer	Date of Assessment	Author
Medications brought by patient?	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Precaution Necessity Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Chlorhexidine Daily Bath Results	No CHG bathing required	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Patient Status	None of the above	06/23/2025 12:53 AM PDT	Your Scripps Health Team
(RETIRED, READ-ONLY) History of MDRO	Greater Than 4 Months	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Patient has an Open Draining Wound that cannot be contained by dressing	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team

C.difficile Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	No Active Diarrhea	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Risks Present	None of the Above	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Results	No C. difficile test necessary	06/23/2025 12:53 AM PDT	Your Scripps Health Team

CRO and Candida auris Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
History of overnight hospitalization and/or invasive procedure outside of the United States within last 12 months	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team
History of LTAC/SNF/Subacute residency within the last 12 months	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Direct transfer or history of hospitalization in last 12mos from outside San Diego county healthcare facility (i.e. ACH, SNF, LTAC)	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Known History of CRO (Carbapenem-resistant organism)	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team
CRO Results and Interventions	Specimen not required.	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Known History of Candida auris	No	06/23/2025 12:53 AM PDT	Your Scripps Health Team
Candida auris Results and Interventions	Specimen not required.	06/23/2025 12:53 AM PDT	Your Scripps Health Team

Departure Condition

Question	Answer	Date of Assessment	Author
DC DISPO & LOCATION (RN)	Acute Care Hospital	06/23/2025 3:28 AM PDT	Your Scripps Health Team

Mental Status - documented in this encounter

Prehospital Treatment

Question	Answer	Entry Date	Author
Prehospital Treatment	No	06/22/2025 10:13 PM PDT	Your Scripps Health Team

Oxygen Therapy

Question	Answer	Entry Date	Author
Patient Activity	At rest	06/22/2025 10:16 PM PDT	Your Scripps Health Team
Pulse Oximetry Type	Intermittent	06/22/2025 10:16 PM PDT	Your Scripps Health Team
Oxygen Therapy	None (Room air)	06/23/2025 3:28 AM PDT	Your Scripps Health Team

Morse Fall Risk

Question	Answer	Entry Date	Author
History of Falling	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Secondary Diagnosis	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Ambulatory Aids	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Intravenous Therapy/Heparin/Saline Lock	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Gait/Transferring	10	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Mental Status	0	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Morse Fall Risk Score	10	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Columbia Suicide Severity Rating Scale

Question	Answer	Entry Date	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	06/22/2025 10:17 PM PDT	Your Scripps Health Team
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	06/22/2025 10:17 PM PDT	Your Scripps Health Team
2. In the past month have you actually had any thoughts of killing yourself?	No	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Violence Risk Screening (Step 1)

Question	Answer	Entry Date	Author
Automatic Triggers	NA	06/22/2025 10:17 PM PDT	Your Scripps Health Team
Risk Factor Triggers	NA	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Entry Date	Author
Violence Screening Outcome	Negative Violence Risk Screen	06/22/2025 10:17 PM PDT	Your Scripps Health Team

Screening

Question	Answer	Entry Date	Author
Where was pt living / residing	Homeless, unsheltered	06/22/2025 10:18 PM PDT	Your Scripps Health Team

Pain Assessment

Question	Answer	Entry Date	Author
Pain Orientation	Left;Lower	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Descriptors	Aching	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Onset	Ongoing	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Frequency	Constant/ continuous	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Patient's Stated Pain Goal	No pain	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Type	Acute pain	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Clinical Progression of Pain	Not changed	06/22/2025 10:20 PM PDT	Your Scripps Health Team
Pain Score	0/No SX - No pain	06/23/2025 12:49 AM PDT	Your Scripps Health Team
Pain Assessment	No/denies pain	06/23/2025 3:28 AM PDT	Your Scripps Health Team
Pain Location	Leg	06/22/2025 10:20 PM PDT	Your Scripps Health Team

Peripheral Vascular

Question	Answer	Entry Date	Author
LUE Edema	Non-pitting	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Entry Date	Author
Skin Color	Appropriate for ethnicity	06/22/2025 10:43 PM PDT	Your Scripps Health Team
Type of Wound (LDA)	Wound	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Endocrine

Question	Answer	Entry Date	Author
Endocrine (WDL)	WDL	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Psychosocial

Question	Answer	Entry Date	Author
Psychosocial (WDL)	WDL	06/22/2025 10:43 PM PDT	Your Scripps Health Team

Departure Condition

Question	Answer	Entry Date	Author
DC DISPO & LOCATION (RN)	Acute Care Hospital	06/23/2025 3:28 AM PDT	Your Scripps Health Team

Medications at Time of Discharge - documented as of this encounter

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.					
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3		08/14/2024	
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024	
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3		10/17/2024	11/03/2025
metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	TAKE 1 TABLET (25 MG TOTAL) BY MOUTH DAILY.	90 tablet			04/10/2025	11/04/2025
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3		08/14/2024	11/02/2025
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3		08/14/2024	11/02/2025

ED Notes - documented in this encounter

Miguel Mendoza Navarro, MD - 06/22/2025 10:10 PM PDT

Formatting of this note might be different from the original.

Emergency Department Record
SCRIPPS MERCY HOSPITAL, CHULA VISTA

Date of Service: 6/22/2025
**Name: **Elizabeth Tina Petruccio
**MRN: **810760324
CSN: 10182838593
**DOB: **7/29/1964 **Age: **60 y.o.
PCP: No primary care provider on file.

[table#1]:
CHIEF COMPLAINT

Leg Swelling (Pt c/o left leg swelling. Pt states she was kicked last Thursday and the next day she was seen in mexico were ultrasound was done and she was prescribed antibiotics. Pt's left lower leg is swollen and black.)

[table#2]:
HISTORY OF THE PRESENT ILLNESS

Elizabeth Tina Petruccio is a 60 y.o. female with h/o a fib, on anticoagulation with eliquis, CHF, EF < 35%, asthma, HTN, presenting to the ED for evaluation of a wound on her left lower extremity. The patient states that she was kicked in the front of her left lower extremity just over a week ago, and since then she started noticing increased pain and swelling with blisters forming. The patient reports that she went to get an ultrasound in Mexico where this happened, and was told that she had a large hematoma. She has in the anterior aspect of the leg has been less swelling, and the blisters have now disappeared, however she has noticed increased formation of the wound on the medial aspect of the left lower extremity. She has continued to bear weight. She reports that she ran out of her Eliquis about a week ago. She has any fevers.

Independent Historian: The patient herself

External Notes Reviewed: Discharge summary on December 20, 2024

[table#3]:
PAST MEDICAL HISTORY

has a past medical history of Acute osteomyelitis of cervical spine (CMS/HCC) (01/26/2024), Asthma, Atrial fibrillation (CMS/HCC), Atrial fibrillation with rapid ventricular response (CMS/HCC) (01/13/2024), Calculus of gallbladder without cholecystitis without obstruction (02/18/2024), CHF (congestive heart failure) (CMS/HCC), and Empyema of left pleural space (CMS/HCC) (01/26/2024).

[table#4]:
PAST SURGICAL HISTORY

has no past surgical history on file.

[table#5]:
ALLERGIES

Allergies

[table#6]:
MEDICATIONS

[table#7]:
Current Outpatient Medications
Medication	Instructions
•	Advair HFA 115-21 mcg/actuation inhaler	2 puffs, inhalation, 2 times daily
•	albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	2 puffs, inhalation, Every 4 hours PRN, Caloptima Mcal ID 93931022D30310
•	ammonium lactate (LAC-HYDRIN) 12 % lotion	topical, 2 times daily
•	apixaban (ELIQUIS)	5 mg, oral, 2 times daily
•	benzonatate (TESSALON)	100 mg, oral, Every 8 hours
•	furosemide (LASIX)	40 mg, oral, Daily
•	gabapentin (NEURONTIN)	600 mg, oral, 3 times daily
•	metoprolol succinate XL (TOPROL-XL)	25 mg, oral, Daily
•	multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	1 tablet, oral, Daily
•	potassium chloride (MICRO-K) 10 mEq CR capsule	10 mEq, oral, Daily
•	predniSONE (DELTASONE)	40 mg, oral, Daily with breakfast
•	sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	1 tablet, oral, 2 times daily

[table#8]:
SOCIAL HISTORY

[table#9]:
Social History

[table#10]:
Substance and Sexual Activity
Alcohol Use	Not Currently

Tobacco Use History

[table#11]:
Social History

[table#12]:
Substance and Sexual Activity
Drug Use	Not Currently

[table#13]:
PHYSICAL EXAM

[table#14]:
ED Triage Vitals [06/22/25 2216]
Temp	Heart Rate	Resp	BP	SpO2
36.6 °C (97.9 °F)	**99**	**16**	**98/65**	**97 %**
Temp src	Heart Rate Source	Patient Position	BP Location	FiO2 (%)
*

*--** | **--** | **--** | **--** | **--** |

BP 104/68 | Pulse 80 | Temp 36.6 °C (97.9 °F) | Resp 16 | Ht 1.676 m (5' 6") | Wt 65.9 kg (145 lb 4.5 oz) | LMP (LMP Unknown) | SpO2 95% | BMI 23.45 kg/m^2

PHYSICAL EXAM
General: Appears well. In no severe distress.
HEENT: Normocephalic, atraumatic; PERRLA, EOMI. Moist mucous membranes.
Neck: Supple, no meningismus.
Cardiovascular: Regular rate and rhythm. No murmurs.
Pulmonary: Speaking in full sentences. Clear, equal breath sounds bilaterally.
Chest: Nontender, no crepitance.
Gastrointestinal: Soft, non-tender, non-distended; no masses appreciated. Normal bowel sounds.
Back: No costovertebral angle tenderness. No midline tenderness to palpation.
Musculoskeletal: No deformities. 14x15cm area of black eschar over the medial LLE. No discharge. No open wounds. Diffuse firmness to the toes over the eschared lesion. Soft tissues around the lesion, appears to be very soft. She has a normal distal pulse. Tremulous. She has normal sensation to light touch distally. Able to flex and extend ankle without significant pain. There is also mild surrounding erythema surrounding the large eschar.

[image]

Skin: Warm and well perfused. No evidence of cyanosis or petechiae.
Neurological: A & O x 4, moving all extremities. Stable gait.
Psychiatric: No SI or HI.

[table#15]:
**ED COURSE **

[table#16]:
Medications
MORphine 4 mg/mL injection 4 mg (4 mg intraVENOUS Given 6/22/25 2319)
ondansetron (PF) (ZOFRAN) injection 4 mg (4 mg intraVENOUS Given 6/22/25 2325)
cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 1 g (1 g intraVENOUS Given 6/22/25 2323)
linezolid in dextrose 5% (ZYVOX) 600 mg/300 mL IVPB 600 mg (0 mg intraVENOUS Stopped 6/23/25 0059)
sodium chloride 0.9 % bolus 250 mL (0 mL intraVENOUS Stopped 6/22/25 2355)

LABORATORY ANALYSIS
**Labs (all labs /ekg/ reviewed and interpreted by myself unless stated otherwise **
[table#17]:
Labs Reviewed
BASIC METABOLIC PANEL - Abnormal
Result	Value
	Fasting?		
	Glucose	87	
	Sodium	142	
	Potassium	4.7	
	Chloride	106	
	CO2, Total	24	
	Urea Nitrogen	48 (*)	
	Creatinine	2.10 (*)	
	CKD-EPI eGFR 2021	26 (*)	
	Anion Gap	12	
	Osmolality Calculated	306 (*)	
	Calcium	9.1	
	*Narrative: *
	For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
*100-125 mg/dL: Impaired Fasting Glucose *
*>125 mg/dL: Diabetes**

**In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.*
|
| **CBC WITH DIFFERENTIAL - Abnormal** |
	WBC	7.7	
	RBC	2.70 (*)	
	HGB	8.5 (*)	
	HCT	26.2 (*)	

	MCV	97	
	MCH	32	
	MCHC	32.4	
	RDW	12.8	
	Platelet Count	387	
	MPV	8.5 (*)	
	nRBC	0.0	
	Neutrophils %	56.4	
	IG%	0.5	
	Lymphocytes %	27.8	
	Monocytes %	8.5	
	Eosinophils %	6.1	
	Basophils %	0.7	
	Absolute Neutrophils	4.33	
	IG#	0.04	
	Absolute Lymphocytes	2.13	
	Absolute Monocytes	0.65	
	Absolute Eosinophils	0.47	
	Absolute Basophils	0.05	
	Preliminary Abs Neut	4.33	
PROTHROMBIN TIME-INR - Normal
	Protime	10.4	
	INR	0.9	
ACTIVATED PTT - Normal
	aPTT	33	
LACTIC ACID, VENOUS - Normal
	Lactic Acid, Venous	1.20	
BLOOD CULTURE
BLOOD CULTURE
**CBC AND DIFFERENTIAL,
ORDERABLE**
	*Narrative: *
	The following orders were created for panel order CBC with Differential.
*Procedure Abnormality Status *
*--------- ---------- ------ *
*CBC with Differential[921618312] Abnormal Final result *

Please view results for these tests on the individual orders. |
| **RAINBOW DRAW** |
| | *Narrative: * |
| | *The following orders were created for panel order Rainbow draw.*
*Procedure Abnormality Status *
*--------- ---------- ------ *
*Gold Top[921618854] Final result *

Please view results for these tests on the individual orders. |
| **GOLD TOP** |
| | Extra Tube | Hold for add-ons. | |
| **EXTRA TUBES** |
| | *Narrative: * |
| | *The following orders were created for panel order Extra Tubes.*
*Procedure Abnormality Status *
*--------- ---------- ------ *
*Gold Top[921619672] Final result *

Please view results for these tests on the individual orders. |
| **GOLD TOP** |
| | Extra Tube | Hold for add-ons. | |

IMAGING
**Imaging:(all imaging studies reviewed and interpreted by myself unless stated otherwise **
[table#18]:
CT lower extremity left without contrast
Preliminary Result
IMPRESSION:
Large subcutaneous hematoma in the left mid calf measuring up to 8 cm. No acute osseous abnormality.

| release |
| |
| X-ray tibia and fibula left 1 or 2 views |
| **ED Interpretation** |
| No fracture or dislocation. Diffuse medial soft tissue edema/collection. |
| |

Procedures

[table#19]:
ED Course as of 06/23/25 0308
Sun Jun 22, 2025
2248	Case discussed with orthopedics, Dr. Rosenfield, who recommends evaluation by plastic surgery and/or general surgery given that the wound probably need to be debrided. [MM]
2300	Attempted to contact plastics at the Scripps Mercy, but was unable to get a hold of on-call physician, Dr. Rodriguez. I left a voicemail for call back. [MM]
2304	X-ray of the left tib-fib without evidence of acute fracture or dislocation. Diffuse soft tissue edema, within the medial aspect of the leg as interpreted by myself. [MM]
2311	Chronic appearing anemia. No indication for transfusion at this time. Appears to be at her baseline. [MM]
2312	Considered but doubt compartment syndrome. Consider cellulitis, deep space abscess, necrotizing fasciitis, large hematoma, fracture. [MM]
Mon Jun 23, 2025
0103	Hematoma appears to be about 8 x 15 cm [MM]
0111	On reevaluation, the patient is noted to be resting comfortably, in no acute distress [MM]
ED Course User Index
[MM] Mendoza Navarro, Miguel Angel, MD

CRITICAL CARE NOTE:
The high probability of sudden clinically significant deterioration of the patient's condition required the highest level of preparedness to intervene urgently. I provided critical care services in the following manner, chart data review, documentation time, medication orders and management, as well as review and ordering laboratory and radiographic studies. Total critical care time was found to be 55 minutes, not including time spent with other patients, excluding other separate billable procedures.

[table#20]:
MEDICAL DECISION MAKING

Elizabeth Tina Petruccio is a 60 y.o. female presenting to the ED with increased pain over large area of black es
char throughout her left calf due to a large subcutaneous hematoma which measures at least 8 x 15 cm. She has normal pulses distally, normal sensation to light touch distally. No crepitus appreciated. Considered but doubt compartment syndrome or necrotizing fasciitis. Symptoms likely associated to the very large subcutaneous hematoma. Fortunately, the patient ran out of her Eliquis about a week ago. Nontoxic or septic appearing. Afebrile with evidence of tachycardia or hypotension with no evidence of leukocytosis with a normal lactic acid. Case was initially discussed with orthopedic surgery here, but recommended transferring for possible plastics evaluation. The case was discussed with UCSD trauma, and accepted for transfer.

The patient has a high level of complexity due to these comorbid/chronic medical conditions: Complex presentation, recent left lower extremity trauma, with chronic anticoagulation.

Discussion of Management with Additional Providers and Consultants: UCSD trauma

Patient's care may be negatively impacted by access to care. Based on these, patient will have barriers to follow up in a timely manner.

[table#21]:
IMPRESSION

[table#22]:
Final diagnoses:
[S80.12XA] Leg hematoma, left, initial encounter (Primary)
[M79.605] Pain of left lower extremity

[table#23]:
PLAN

DISPOSITION:

Transfer to another facility UCSD trauma

Part of this note was completed using Nuance dictation system which could lead to errors during speech to text transcription.

Mendoza Navarro, Miguel Angel, MD
06/23/25 0239

Allergies
[table#24]:
Allergen	Reactions
•	Clindamycin	Other (see comments)
		*Patient unsure of what reaction she has from medication. *
•	Lactose	Other (see comments)
		Patient having diarrhea; only fluid milk - all other lactose items ok

Social History
[table#25]:
Tobacco Use
Smoking Status	Former
•	Passive exposure:	Past
Smokeless Tobacco	Never

Mendoza Navarro, Miguel Angel, MD
06/23/25 0308

Electronically signed by Miguel Mendoza Navarro, MD at 06/23/2025 2:39 AM PDT
Electronically signed by Miguel Mendoza Navarro, MD at 06/23/2025 3:08 AM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
Pneumococcal 50+ (1 of 2 - PCV)	**07/29/1983**		
Breast Cancer Screening	**07/29/2004**		
Colorectal Cancer Screening	**07/29/2009**		
RSV (1 - Risk 50-74 years 1-dose series)	**07/29/2014**		
Shingrix Vaccine (1 of 2)	**07/29/2014**		
Cervical Cancer Screening	**02/27/2024**	02/27/2019, 01/20/2012	
Flu Shot (#1)	**09/01/2026**	11/20/2012	
Adult Tdap/Td Vaccines (2 - Td or Tdap)	02/09/2034	02/09/2024	

Results - documented in this encounter

CT lower extremity left without contrast (CT LOWER EXTREMITY LEFT WITHOUT CONTRAST) - Final result (06/23/2025 12:30 AM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities	Left	Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			06/23/2025 12:38 AM PDT	

Impressions

06/23/2025 10:12 AM PDT
IMPRESSION:
Large subcutaneous hematoma in the left mid calf measuring up to 15.8 cm. No acute osseous abnormality.

release

Narrative

06/23/2025 10:12 AM PDT
EXAMINATION: CT LOWER EXTREMITY LEFT WITHOUT CONTRAST

DATE/TIME: 6/23/2025 12:30 AM

HISTORY: L lower extremity pain/swelling

COMPARISON: Radiographs from 6/22/2025

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:
EXTREMITY: Left lower leg
Mild diffuse subcutaneous edema.
There is a large blood attenuation fluid collection in the subcutaneous soft tissues of the left mid calf measuring approximately 4.3 x 8.1 x 15.8 cm
No acute fracture

EXPOSURE:
This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 2; Series: 7; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 9; DLP: 202
Exposure: 1; Series: 4; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 8; DLP: 529

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

Procedure Note

Jeffrey Rudie, MD, PhD - 06/23/2025

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: CT LOWER EXTREMITY LEFT WITHOUT CONTRAST

DATE/TIME: 6/23/2025 12:30 AM

HISTORY: L lower extremity pain/swelling

COMPARISON: Radiographs from 6/22/2025

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:
EXTREMITY: Left lower leg
Mild diffuse subcutaneous edema.
There is a large blood attenuation fluid collection in the subcutaneous soft tissues of the left mid calf measuring approximately 4.3 x 8.1 x 15.8 cm
No acute fracture

EXPOSURE:
This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 2; Series: 7; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 9; DLP: 202
Exposure: 1; Series: 4; Anatomy: Lower extremities; Phantom: 32 cm; CTDIvol: 8; DLP: 529

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

IMPRESSION:
IMPRESSION:
Large subcutaneous hematoma in the left mid calf measuring up to 15.8 cm. No acute osseous abnormality.

release

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	IMG CT PROCEDURES	Final Result

X-ray tibia and fibula left 1 or 2 views (X-RAY TIBIA AND FIBULA LEFT 1 OR 2 VIEWS) - Final result (06/22/2025 11:03 PM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities, Lower Leg	Left	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			06/23/2025 8:05 AM PDT	

Impressions

06/23/2025 8:06 AM PDT
IMPRESSION: No fracture or periosteal process appreciated.
2. Soft tissue lucency in the calf consistent with fluid collection

release

Narrative

06/23/2025 8:06 AM PDT

Narrative

EXAMINATION: X-RAY TIBIA AND FIBULA LEFT 1 OR 2 VIEWS

DATE/TIME:6/22/2025 11:03 PM

HISTORY: LLE pain

COMPARISON: None.

FINDINGS:

BONE: No apparent bony abnormality.
SOFT TISSUE: Soft tissue lucency in the posterior calf region
OTHER: Negative.

Procedure Note

Trevor Nelson, MD - 06/23/2025
Formatting of this note might be different from the original.
EXAMINATION: X-RAY TIBIA AND FIBULA LEFT 1 OR 2 VIEWS

DATE/TIME:6/22/2025 11:03 PM

HISTORY: LLE pain

COMPARISON: None.

FINDINGS:

BONE: No apparent bony abnormality.
SOFT TISSUE: Soft tissue lucency in the posterior calf region
OTHER: Negative.

IMPRESSION:
IMPRESSION: No fracture or periosteal process appreciated.
2. Soft tissue lucency in the calf consistent with fluid collection

release

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CULTURE, BLOOD (BLOOD CULTURE) - Final result (06/22/2025 10:52 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			06/28/2025 12:01 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of left upper limb / Unknown	Venipuncture / Unknown	06/22/2025 10:52 PM PDT	06/22/2025 11:03 PM PDT

Narrative

MERCY LAB CV - 06/28/2025 12:01 AM PDT
Reference range: No growth

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	7.7	3.4 - 11.0 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
RBC	2.70	3.98 - 5.25 M/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
HGB	8.5	11.9 - 15.3 g/dL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
HCT	26.2	37.3 - 46.7 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MCV	97	81 - 100 fL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MCH	32	26 - 33 pg		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MCHC	32.4	31.0 - 36.0 g/dL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
RDW	12.8	11.7 - 14.9 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Platelet Count	387	150 - 425 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
MPV	8.5	9.0 - 12.8 fL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Neutrophils %	56.4	38.0 - 74.0 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
IG%	0.5	0.0 - 1.2 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Lymphocytes %	27.8	16.0 - 48.0 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Monocytes %	8.5	4.9 - 12.5 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Eosinophils %	6.1	0.0 - 9.5 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Neutrophils	4.33	1.50 - 7.40 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	2.13	0.90 - 3.10 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Monocytes	0.65	0.26 - 0.87 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Eosinophils	0.47	0.00 - 0.51 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	
Preliminary Abs Neut	4.33	1.50 - 7.40 K/mcL		06/22/2025 11:11 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTIC ACID, SERUM (LACTIC ACID, VENOUS) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Lactic Acid, Venous	1.20	0.70 - 2.10 mmol/L		06/22/2025 11:13 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:00 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			06/28/2025 12:01 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous line / Unknown		06/22/2025 10:49 PM PDT	06/22/2025 11:03 PM PDT

Narrative

```
MERCY LAB CV - 06/28/2025 12:01 AM PDT
Reference range: No growth
```

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

APTT (ACTIVATED PTT) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
aPTT	33	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds		06/22/2025 11:32 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTIME-INR (PROTHROMBIN TIME-INR) - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protime	10.4	10.0 - 13.1 Seconds		06/22/2025 11:32 PM PDT	MERCY LAB CV	
INR	0.9	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation		06/22/2025 11:32 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (06/22/2025 10:49 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				06/22/2025 11:24 PM PDT	MERCY LAB CV	
Glucose	87	70 - 125 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Sodium	142	137 - 145 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Potassium	4.7	3.5 - 5.1 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Chloride	106	98 - 107 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
CO2, Total	24	22 - 30 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Urea Nitrogen	48	7 - 17 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Creatinine	2.10	0.50 - 1.00 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	
CKD-EPI eGFR 2021	26	>60 mL/min/1.73m*2		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Anion Gap	12	6 - 14 mmol/L		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Osmolality Calculated	306	280 - 305 mOsm/kg H20		06/22/2025 11:24 PM PDT	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		06/22/2025 11:24 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	06/22/2025 10:49 PM PDT	06/22/2025 11:01 PM PDT

Narrative

MERCY LAB CV - 06/22/2025 11:24 PM PDT
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Miguel Mendoza Navarro MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

Visit Diagnoses - documented in this encounter

Diagnosis
Leg hematoma, left, initial encounter - Primary
Left leg pain

Administered Medications - documented in this encounter

Inactive Administered Medications - up to 3 most recent administrations

Medication Order	MAR Action	Action Date	Dose	Rate	Site
cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 1 g 1 g, intraVENOUS, at 240 mL/hr, Administer over 5 Minutes, Once, On Sun 6/22/25 at 2330, For 1 dose, Refrigerate. Administer IV push up to 2 g over 5 minutes., Indications: skin and skin structure infection	Given	06/22/2025 11:23 PM PDT	1 g	240 mL/hr	
linezolid in dextrose 5% (ZYVOX) 600 mg/300 mL IVPB 600 mg 600 mg, intraVENOUS, at 300 mL/hr, Administer over 60 Minutes, Once, On Sun 6/22/25 at 2315, For 1 dose, `Premix. Flush line before and after administration, Indications: skin and skin structure infection	New Bag	06/22/2025 11:41 PM PDT	600 mg	300 mL/hr	
MORphine 4 mg/mL injection 4 mg 4 mg, intraVENOUS, Once, On Sun 6/22/25 at 2300, For 1 dose, `	Given	06/22/2025 11:19 PM PDT	4 mg		
MORphine 4 mg/mL injection 4 mg 4 mg, intraVENOUS, Once, On Mon 6/23/25 at 0345, For 1 dose, `	Given	06/23/2025 3:21 AM PDT	4 mg		
ondansetron (PF) (ZOFRAN) injection 4 mg 4 mg, intraVENOUS, Once, On Sun 6/22/25 at 2300, For 1 dose, Individual doses of 8 mg or less may be given as IV push over 2-3 minutes `	Given	06/22/2025 11:25 PM PDT	4 mg		
sodium chloride 0.9 % bolus 250 mL 250 mL, intraVENOUS, at 500 mL/hr, Administer over 30 Minutes, Once, On Sun 6/22/25 at 2330, For 1 dose, `Premix	New Bag	06/22/2025 11:25 PM PDT	250 mL	500 mL/hr	

Insurance - documented as of this encounter

Guarantor: Petruccio, Elizabeth Tina

Account Type	Relation to Patient	Date of Birth	Phone	Billing Address
Personal/Family	Self	07/29/1964		HOMELESS SAN DIEGO, CA 99999

MEDI-CAL

Member	Subscriber	Plan / Payer (Effective 05/01/2025-Present)
Name: Petruccio, Elizabeth Tina Member ID: 93931022D Relation to Subscriber: Self	Name: Petruccio, Elizabeth Tina Subscriber ID: 93931022D	Payer ID: Not on file Group ID: Not on file Type: Medi-Cal Phone: 916-636-1200 Address: PO BOX 15700 SACRAMENTO, CA 95852-1700

Advance Directives

For more information, please contact: 858-927-7132 (6AM - 4PM America/Los_Angeles, Monday-Friday)

Full Code (Latest Code Status on File)

Date Activated	Date Inactivated	Comments
1/12/2024 9:06 PM	2/17/2024 6:01 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:20 PM	12/23/2021 8:29 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:09 PM	12/22/2021 10:20 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers

Document Coverage Dates

Jun. 22, 2025 - Jun. 23, 2025

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

Encounter Providers

Encounter Date

Miguel Mendoza Navarro, MD (Attending)
NPI: 1619293495
619-691-7290 (Work)
619-691-7435 (Fax)
435 H St
Chula Vista, CA 91910-4307
Emergency Medicine

Jun. 22, 2025 - Jun. 23, 2025

Elizabeth Tina Petruccio

Summary of Care, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
12/26/2024 3:45 PM PST	Telemedicine	San Diego Cardiac Care 4060 4th Ave Suite 650 SAN DIEGO, CA 92103-2118 858-900-9778	**Jerrold Glassman, MD** 4060 Fourth Ave #650 San Diego, CA 92103 619-819-7222 (Work) 619-299-5023 (Fax)	

Allergies - documented as of this encounter (statuses as of 10/13/2026)

Active Allergy	Reactions	Criticality	Noted Date	Comments
Clindamycin	Other (see comments)		09/23/2024	Patient unsure of what reaction she has from medication.
Lactose	Other (see comments)	Low	01/20/2024	Patient having diarrhea; only fluid milk - all other lactose items ok

Inactive Allergy	Reactions	Criticality	Noted Date	Inactivity Date	Comments
Lincosamides	Other (see comments), Unknown Reaction		07/11/2012	09/23/2024	

Medications - documented as of this encounter (statuses as of 10/13/2026)

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.						Active
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3		08/14/2024		Active
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024		Active
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3		08/14/2024	11/02/2025	Discontinued
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3		08/14/2024	11/02/2025	Discontinued
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3		10/17/2024	11/03/2025	Discontinued

Active Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date
Gallstones	02/18/2024	
Gall bladder pain	02/18/2024	
Pus collection in space between lung and chest wall	01/26/2024	
Acute osteomyelitis of cervical spine	01/26/2024	
Heart failure	01/13/2024	
Loculated pleural effusion	01/13/2024	
Atrial fibrillation (irregular heartbeat)	01/13/2024	
Respiratory insufficiency	01/12/2024	
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Resolved Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date	Resolved Date
Severe sepsis	02/18/2024		03/24/2024

Immunizations - documented as of this encounter

Immunization	Administration Dates	Next Due
Tdap greater than or equal to 7yrs IM	02/09/2024	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Passive Smoke Exposure: Past				
Smokeless Tobacco: Never				

Patient History	Answer	Date Recorded
Alcohol Use Status	Not Currently	04/18/2024
Oz/Week (Alcohol Misuse Male >= 9 OR Female >= 4.8)	Not on file	04/18/2024

Housing Stability	Answer	Date Recorded
What is your living situation today?	Do not have a stable place to live	01/19/2024

Food Insecurity	Answer	Date Recorded
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024

Transportation Needs	Answer	Date Recorded
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024

Pregnant	Comments
No	

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

Last Filed Vital Signs - documented in this encounter

Not on file

Progress Notes - documented in this encounter

Jerrold Glassman, MD - 12/26/2024 3:45 PM PST

Formatting of this note might be different from the original.

Telephonic CARDIOLOGY PROGRESS NOTE
PCP: No primary care provider on file.
REFERRING PROVIDER: No ref. provider found
CHIEF COMPLAINT / REASON FOR VISIT : Hospital follow-up

HISTORY OF PRESENT ILLNESS:
Elizabeth Tina Petruccio is a very pleasant 61 y.o. female with initial presentation at hospital being left leg swollen. She has a history of atrial fibrillation on Eliquis with a low ejection fraction of 35%. She observed as she had a large hematoma.

She has a history of osteomyelitis, atrial fibrillation, cholecystitis, heart failure, and empyema.

Post hospital stay she is doing satisfactory.

Her medical provider is UCSD. She was advised to see them at the beginning of the year.

She maintains sinus rhythm. Echocardiogram does show depressed EF of 25%.
PAST MEDICAL HISTORY:
[table#1]:
	Diagnosis	Plan
1.	**Essential hypertension **	
2.	Shortness of breath	
3.	History of noncompliance with medical treatment	

Current Medications

Allergies

** Review of Systems**
12 point review of systems aside from pertinent positives covered in Subjective and Problem List, otherwise negative.

** PHYSICAL EXAM **

Labs/ Studies :

***BMP:* **
[table#2]:
Lab Results
Component	Value	Date/Time
	GLUCOSE	87	06/22/2025 10:49 PM
	CALCIUM	9.1	06/22/2025 10:49 PM
	NA	142	06/22/2025 10:49 PM
	K	4.7	06/22/2025 10:49 PM
	CO2	24	06/22/2025 10:49 PM
	CL	106	06/22/2025 10:49 PM
	BUN	48 (H)	06/22/2025 10:49 PM
	CREATININE	2.10 (H)	06/22/2025 10:49 PM

Lipids:
No results found for: "CHOL"
[table#3]:
Lab Results
Component	Value	Date/Time
	HDL	33 (L)	01/13/2024 05:14 AM

[table#4]:
Lab Results
Component	Value	Date/Time

| | LDLCALC | 63 | 01/13/2024 05:14 AM |

[table#5]:
Lab Results
Component	Value	Date/Time
	TRIG	72	01/13/2024 05:14 AM

Assessment/Plan :

Multiple medical issues but stable.

Does have a history of polysubstance drug abuse and noncompliance

***Follow Up **: *

Unfortunately she has a history of medical noncompliance and this would adversely affect her

Electronically signed by ***Jerrold Glassman, MD September 17, 2025*** 11:37 AM

[table#6]:
Current Outpatient Medications
Medication	Sig	Dispense	Refill
•	Advair HFA 115-21 mcg/actuation inhaler	Inhale 2 puffs 2 (two) times a day.		
•	albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	INHALE 2 PUFFS EVERY 4 (FOUR) HOURS AS NEEDED FOR WHEEZING. CALOPTIMA MCAL ID 93931022D30310	90 g	3
•	ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.		
•	apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3
•	benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule	0
•	furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3
•	gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule	0
•	metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	TAKE 1 TABLET (25 MG TOTAL) BY MOUTH DAILY.	90 tablet	0
•	multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily. (Patient not taking: Reported on 9/18/2024)	30 tablet	0
•	potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3
•	predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet	0
•	sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3

[table#7]:
No current facility-administered medications for this visit.

Allergies
[table#8]:
Allergen	Reactions
•	Clindamycin	Other (see comments)
		*Patient unsure of what reaction she has from medication. *
•	Lactose	Other (see comments)
		Patient having diarrhea; only fluid milk - all other lactose items ok

Electronically signed by Jerrold Glassman, MD at 09/17/2025 3:14 PM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
Pneumococcal 50+ (1 of 2 - PCV)	**07/29/1983**		
Breast Cancer Screening	**07/29/2004**		
Colorectal Cancer Screening	**07/29/2009**		
RSV (1 - Risk 50-74 years 1-dose series)	**07/29/2014**		
Shingrix Vaccine (1 of 2)	**07/29/2014**		
Cervical Cancer Screening	**02/27/2024**	02/27/2019, 01/20/2012	
Flu Shot (#1)	**09/01/2026**	11/20/2012	
Adult Tdap/Td Vaccines (2 - Td or Tdap)	02/09/2034	02/09/2024	

Results - documented in this encounter

Not on file

Visit Diagnoses - documented in this encounter

Diagnosis
High blood pressure - Primary
History of noncompliance with medical treatment
Heart failure
History of pleural empyema

Advance Directives

For more information, please contact: 858-927-7132 (6AM - 4PM America/Los_Angeles, Monday-Friday)

Full Code (Latest Code Status on File)

Date Activated	Date Inactivated	Comments
1/12/2024 9:06 PM	2/17/2024 6:01 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:20 PM	12/23/2021 8:29 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:09 PM	12/22/2021 10:20 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers

Document Coverage Dates

Dec. 26, 2024

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

Encounter Providers

Encounter Date

Jerrold Glassman, MD (Attending)
NPI: 1245326883
619-819-7222 (Work)
619-299-5023 (Fax)
4060 Fourth Ave #650
San Diego, CA 92103
Cardiovascular Disease

Dec. 26, 2024

Elizabeth Tina Petruccio

Summary of Care, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Reason for Visit

Reason
Blood Pressure Check

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
12/24/2024 12:34 AM PST - 12/24/2024 4:05 AM PST	Emergency	Scripps Mercy Hospital Chula Vista Emergency Department 435 H Street Chula Vista, CA 91910-4307 619-691-7000	**Douglas Ballesteros, MD** 435 H St Chula Vista, CA 91910-4307 619-691-7290 (Work) 619-691-7435 (Fax)	Discharge Disposition: Disch to Home (Routine)

Allergies - documented as of this encounter (statuses as of 10/13/2026)

Active Allergy	Reactions	Criticality	Noted Date	Comments
Clindamycin	Other (see comments)		09/23/2024	Patient unsure of what reaction she has from medication.
Lactose	Other (see comments)	Low	01/20/2024	Patient having diarrhea; only fluid milk - all other lactose items ok

Inactive Allergy	Reactions	Criticality	Noted Date	Inactivity Date	Comments
Lincosamides	Other (see comments), Unknown Reaction		07/11/2012	09/23/2024	

Medications - documented as of this encounter (statuses as of 10/13/2026)

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.						Active
ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.				02/16/2024		Active
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			02/17/2024		Active
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3		08/14/2024		Active
gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule			09/18/2024		Active
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule			09/23/2024		Active
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024		Active
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/ actuation inhaler	INHALE 2 PUFFS EVERY 4 (FOUR) HOURS AS NEEDED FOR WHEEZING. CALOPTIMA MCAL ID 93931022D30310	90 g	3		10/15/2024		Active
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3		08/14/2024	11/02/2025	Discontinued
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3		08/14/2024	11/02/2025	Discontinued
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3		10/17/2024	11/03/2025	Discontinued
metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	TAKE 1 TABLET (25 MG TOTAL) BY MOUTH DAILY.	90 tablet			12/20/2024	04/10/2025	Discontinued

Active Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date
Gallstones	02/18/2024	
Gall bladder pain	02/18/2024	
Pus collection in space between lung and chest wall	01/26/2024	
Acute osteomyelitis of cervical spine	01/26/2024	
Heart failure	01/13/2024	
Loculated pleural effusion	01/13/2024	
Atrial fibrillation (irregular heartbeat)	01/13/2024	
Respiratory insufficiency	01/12/2024	
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Resolved Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date	Resolved Date
Severe sepsis	02/18/2024		03/24/2024

Immunizations - documented as of this encounter

Immunization	Administration Dates	Next Due
Tdap greater than or equal to 7yrs IM	02/09/2024	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Passive Smoke Exposure: Past				
Smokeless Tobacco: Never				

Patient History	Answer	Date Recorded
Alcohol Use Status	Not Currently	04/18/2024
Oz/Week (Alcohol Misuse Male >= 9 OR Female >= 4.8)	Not on file	04/18/2024

Housing Stability	Answer	Date Recorded
What is your living situation today?	Do not have a stable place to live	01/19/2024

Food Insecurity	Answer	Date Recorded
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024

Transportation Needs	Answer	Date Recorded
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024

Pregnant	Comments
No	

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

Last Filed Vital Signs - documented in this encounter

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	104/79	12/24/2024 3:41 AM PST	
Pulse	98	12/24/2024 3:41 AM PST	
Temperature	36.9 °C (98.4 °F)	12/24/2024 12:32 AM PST	
Respiratory Rate	16	12/24/2024 3:41 AM PST	
Oxygen Saturation	99%	12/24/2024 3:41 AM PST	
Inhaled Oxygen Concentration	-	-	
Weight	71 kg (156 lb 8.4 oz)	12/24/2024 3:42 AM PST	
Height	167.6 cm (5' 6")	12/24/2024 3:42 AM PST	
Body Mass Index	25.26	12/24/2024 3:42 AM PST	

Functional Status - documented as of this encounter

Oxygen Therapy

Question	Answer	Date of Assessment	Author
Patient Activity	At rest	12/24/2024 12:32 AM PST	Your Scripps Health Team
Pulse Oximetry Type	Intermittent	12/24/2024 12:32 AM PST	Your Scripps Health Team
Oxygen Therapy	None (Room air)	12/24/2024 3:41 AM PST	Nurse John B

Height and Weight

Question	Answer	Date of Assessment	Author
Weight	2504.43	12/24/2024 3:42 AM PST	Nurse John B

Tuberculosis Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	Denies symptoms	12/24/2024 12:45 AM PST	Your Scripps Health Team
Risks Present	Denies risks	12/24/2024 12:45 AM PST	Your Scripps Health Team
Results	TB Risk Screen Negative	12/24/2024 12:45 AM PST	Your Scripps Health Team

Pain Assessment

Question	Answer	Date of Assessment	Author
Pain Assessment	No/denies pain	12/24/2024 3:41 AM PST	Nurse John B

Morse Fall Risk

Question	Answer	Date of Assessment	Author
History of Falling	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Secondary Diagnosis	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Ambulatory Aids	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Intravenous Therapy/Heparin/Saline Lock	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Gait/Transferring	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Mental Status	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Morse Fall Risk Score	0	12/24/2024 12:46 AM PST	Your Scripps Health Team

Columbia Suicide Severity Rating Scale

Question	Answer	Date of Assessment	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	12/24/2024 12:46 AM PST	Your Scripps Health Team
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	12/24/2024 12:46 AM PST	Your Scripps Health Team
2. In the past month have you actually had any thoughts of killing yourself?	No	12/24/2024 12:46 AM PST	Your Scripps Health Team

Violence Risk Screening (Step 1)

Question	Answer	Date of Assessment	Author
Automatic Triggers	NA	12/24/2024 12:46 AM PST	Your Scripps Health Team
Risk Factor Triggers	NA	12/24/2024 12:46 AM PST	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Date of Assessment	Author
Violence Screening Outcome	Negative Violence Risk Screen	12/24/2024 12:46 AM PST	Your Scripps Health Team

Screening

Question	Answer	Date of Assessment	Author
Where was pt living / residing	Private Residence	12/24/2024 12:46 AM PST	Your Scripps Health Team

Prehospital Treatment

Question	Answer	Date of Assessment	Author
Transport Service	medic 20	12/24/2024 12:47 AM PST	Your Scripps Health Team
Run #	fs24231028	12/24/2024 12:47 AM PST	Your Scripps Health Team
Prehospital Treatment	Yes	12/24/2024 12:47 AM PST	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Date of Assessment	Author
Skin Color/Condition (WDL)	WDL	12/24/2024 12:47 AM PST	Your Scripps Health Team

Endocrine

Question	Answer	Date of Assessment	Author
Endocrine (WDL)	WDL	12/24/2024 12:47 AM PST	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	12/24/2024 12:47 AM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	12/24/2024 12:47 AM PST	Your Scripps Health Team

Vital Signs

Question	Answer	Date of Assessment	Author
MAP (Monitor)	105	12/24/2024 1:00 AM PST	Your Scripps Health Team

Vital Signs

Question	Answer	Date of Assessment	Author
EKG Rhythm	Sinus Rhythm	12/24/2024 3:41 AM PST	Nurse John B

Sedation

Question	Answer	Date of Assessment	Author
Pasero Opioid-Induced Sedation Scale (POSS)	S	12/24/2024 3:41 AM PST	Nurse John B
Richmond Agitation Sedation Scale (RASS)	0	12/24/2024 3:41 AM PST	Nurse John B

Feature 3: Altered Level of Consciousness

Answer	Date of Assessment	Author
Negative	12/24/2024 3:41 AM PST	Nurse John B

BSA (Calculated - sq m)

Answer	Date of Assessment	Author
1.82	12/24/2024 3:42 AM PST	Nurse John B

Departure Condition

Question	Answer	Date of Assessment	Author
DC DISPO & LOCATION (RN)	Private Residence	12/24/2024 4:02 AM PST	Nurse John B

Oxygen Therapy

Question	Answer	Date of Assessment	Author
Patient Activity	At rest	12/24/2024 12:32 AM PST	Your Scripps Health Team
Pulse Oximetry Type	Intermittent	12/24/2024 12:32 AM PST	Your Scripps Health Team
Oxygen Therapy	None (Room air)	12/24/2024 3:41 AM PST	Nurse John B

Tuberculosis Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	Denies symptoms	12/24/2024 12:45 AM PST	Your Scripps Health Team
Risks Present	Denies risks	12/24/2024 12:45 AM PST	Your Scripps Health Team
Results	TB Risk Screen Negative	12/24/2024 12:45 AM PST	Your Scripps Health Team

Pain Assessment

Question	Answer	Date of Assessment	Author
Pain Assessment	No/denies pain	12/24/2024 3:41 AM PST	Nurse John B

Morse Fall Risk

Question	Answer	Date of Assessment	Author
History of Falling	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Secondary Diagnosis	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Ambulatory Aids	0	12/24/2024 12:46 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Intravenous Therapy/Heparin/Saline Lock	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Gait/Transferring	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Mental Status	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Morse Fall Risk Score	0	12/24/2024 12:46 AM PST	Your Scripps Health Team

Columbia Suicide Severity Rating Scale

Question	Answer	Date of Assessment	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	12/24/2024 12:46 AM PST	Your Scripps Health Team
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	12/24/2024 12:46 AM PST	Your Scripps Health Team
2. In the past month have you actually had any thoughts of killing yourself?	No	12/24/2024 12:46 AM PST	Your Scripps Health Team

Violence Risk Screening (Step 1)

Question	Answer	Date of Assessment	Author
Automatic Triggers	NA	12/24/2024 12:46 AM PST	Your Scripps Health Team
Risk Factor Triggers	NA	12/24/2024 12:46 AM PST	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Date of Assessment	Author
Violence Screening Outcome	Negative Violence Risk Screen	12/24/2024 12:46 AM PST	Your Scripps Health Team

Screening

Question	Answer	Date of Assessment	Author
Where was pt living / residing	Private Residence	12/24/2024 12:46 AM PST	Your Scripps Health Team

Prehospital Treatment

Question	Answer	Date of Assessment	Author
Transport Service	medic 20	12/24/2024 12:47 AM PST	Your Scripps Health Team
Run #	fs24231028	12/24/2024 12:47 AM PST	Your Scripps Health Team
Prehospital Treatment	Yes	12/24/2024 12:47 AM PST	Your Scripps Health Team

HEENT

Question	Answer	Date of Assessment	Author
HEENT (WDL)	WDL	12/24/2024 12:47 AM PST	Your Scripps Health Team

Endocrine

Question	Answer	Date of Assessment	Author
Endocrine (WDL)	WDL	12/24/2024 12:47 AM PST	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	12/24/2024 12:47 AM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	12/24/2024 12:47 AM PST	Your Scripps Health Team

Vital Signs

Question	Answer	Date of Assessment	Author
MAP (Monitor)	105	12/24/2024 1:00 AM PST	Your Scripps Health Team

Vital Signs

Question	Answer	Date of Assessment	Author
EKG Rhythm	Sinus Rhythm	12/24/2024 3:41 AM PST	Nurse John B

Sedation

Question	Answer	Date of Assessment	Author
Pasero Opioid-Induced Sedation Scale (POSS)	S	12/24/2024 3:41 AM PST	Nurse John B
Richmond Agitation Sedation Scale (RASS)	0	12/24/2024 3:41 AM PST	Nurse John B

Feature 3: Altered Level of Consciousness

Answer	Date of Assessment	Author
Negative	12/24/2024 3:41 AM PST	Nurse John B

Departure Condition

Question	Answer	Date of Assessment	Author
DC DISPO & LOCATION (RN)	Private Residence	12/24/2024 4:02 AM PST	Nurse John B

Mental Status - documented in this encounter

Oxygen Therapy

Question	Answer	Entry Date	Author
Patient Activity	At rest	12/24/2024 12:32 AM PST	Your Scripps Health Team
Pulse Oximetry Type	Intermittent	12/24/2024 12:32 AM PST	Your Scripps Health Team
Oxygen Therapy	None (Room air)	12/24/2024 3:41 AM PST	Nurse John B

Pain Assessment

Question	Answer	Entry Date	Author
Pain Assessment	No/denies pain	12/24/2024 3:41 AM PST	Nurse John B

Morse Fall Risk

Question	Answer	Entry Date	Author
History of Falling	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Secondary Diagnosis	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Ambulatory Aids	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Intravenous Therapy/Heparin/Saline Lock	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Gait/Transferring	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Mental Status	0	12/24/2024 12:46 AM PST	Your Scripps Health Team
Morse Fall Risk Score	0	12/24/2024 12:46 AM PST	Your Scripps Health Team

Columbia Suicide Severity Rating Scale

Question	Answer	Entry Date	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	12/24/2024 12:46 AM PST	Your Scripps Health Team
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	12/24/2024 12:46 AM PST	Your Scripps Health Team
2. In the past month have you actually had any thoughts of killing yourself?	No	12/24/2024 12:46 AM PST	Your Scripps Health Team

Violence Risk Screening (Step 1)

Question	Answer	Entry Date	Author
Automatic Triggers	NA	12/24/2024 12:46 AM PST	Your Scripps Health Team
Risk Factor Triggers	NA	12/24/2024 12:46 AM PST	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Entry Date	Author
Violence Screening Outcome	Negative Violence Risk Screen	12/24/2024 12:46 AM PST	Your Scripps Health Team

Screening

Question	Answer	Entry Date	Author
Where was pt living / residing	Private Residence	12/24/2024 12:46 AM PST	Your Scripps Health Team

Prehospital Treatment

Question	Answer	Entry Date	Author
Transport Service	medic 20	12/24/2024 12:47 AM PST	Your Scripps Health Team
Run #	fs24231028	12/24/2024 12:47 AM PST	Your Scripps Health Team
Prehospital Treatment	Yes	12/24/2024 12:47 AM PST	Your Scripps Health Team

Endocrine

Question	Answer	Entry Date	Author
Endocrine (WDL)	WDL	12/24/2024 12:47 AM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Entry Date	Author
Psychosocial (WDL)	WDL	12/24/2024 12:47 AM PST	Your Scripps Health Team

Vital Signs

Question	Answer	Entry Date	Author
MAP (Monitor)	105	12/24/2024 1:00 AM PST	Your Scripps Health Team

Vital Signs

Question	Answer	Entry Date	Author
EKG Rhythm	Sinus Rhythm	12/24/2024 3:41 AM PST	Nurse John B

Sedation

Question	Answer	Entry Date	Author
Pasero Opioid-Induced Sedation Scale (POSS)	S	12/24/2024 3:41 AM PST	Nurse John B
Richmond Agitation Sedation Scale (RASS)	0	12/24/2024 3:41 AM PST	Nurse John B

Feature 3: Altered Level of Consciousness

Answer	Entry Date	Author
Negative	12/24/2024 3:41 AM PST	Nurse John B

Departure Condition

Question	Answer	Entry Date	Author
DC DISPO & LOCATION (RN)	Private Residence	12/24/2024 4:02 AM PST	Nurse John B

Discharge Instructions - documented in this encounter

Discharge Instructions
Douglas Ballesteros, MD - 12/24/2024 3:44 AM PST
Formatting of this note might be different from the original.
Continue with your previous medications as instructed. The midodrine that you are taking is to keep her blood pressure up at a normal range. If your blood pressure gets up to the 130s systolic that means you do not need to take that medication at that time. Follow closely with your primary physician and cardiologist. Return for any chest pain or worsening of symptoms.
Electronically signed by Douglas Ballesteros, MD at 12/24/2024 3:44 AM PST
Electronically signed by Douglas Ballesteros, MD at 12/24/2024 3:44 AM PST

Attachments
The following attachments were made available to the patient upon discharge.
Low Blood Pressure, All Causes (English)

Medications at Time of Discharge - documented as of this encounter

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.					
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3		08/14/2024	
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024	
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3		10/17/2024	11/03/2025
metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	TAKE 1 TABLET (25 MG TOTAL) BY MOUTH DAILY.	90 tablet			12/20/2024	04/10/2025
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3		08/14/2024	11/02/2025
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3		08/14/2024	11/02/2025

ED Notes - documented in this encounter

Douglas Ballesteros, MD - 12/24/2024 12:27 AM PST
Formatting of this note might be different from the original.
[image]

Date of Service: 12/24/2024
Name: Elizabeth Tina Petruccio
MRN: 810760324
CSN: 10169432593
DOB: 7/29/1964 Age: 60 y.o.

**Chief Complaint: **
[table#1]:
Chief Complaint
Patient presents with
•	Blood Pressure Check
		Patient bib ems after she arrived from a trip to new jersey where she reports having some medications changes. She states she believed that she was supposed so seek medical attention when her systolic BP was below 130. Patient states her lowest BP reading was 114 systolic. Patient denies new complaints, but states ems told her she may be in afib. Patient reports extensive cardiac history and "just wants to do the right thing"

**HPI: **
Elizabeth Tina Petruccio is a 60 y.o. female who presents with concerns about elevated heart rate. She is also unsure what to do about her blood pressure. Patient recently had an prolonged admission in New Jersey. At that time they had told her to use midodrine but hold it if her blood pressure was over 130. This had a very confused so she is seeking confirmation as to how to address these medications. She is also concerned as her heart rate monitor has been running somewhat high. She does have a history of atrial fibrillation. She does not have any current chest pain or shortness of breath. No fevers or chills. She is currently taking ciprofloxacin for recent urinary tract infection.

Past Medical History
Patient has a past medical history of Acute osteomyelitis of cervical spine (CMS/HCC) (01/26/2024), Asthma, Atrial fibrillation (CMS/HCC), Atrial fibrillation with rapid ventricular response (CMS/HCC) (01/13/2024), Calculus of gallbladder without cholecystitis without obstruction (02/18/2024), CHF (congestive heart failure) (CMS/HCC), and Empyema of left pleural space (CMS/HCC) (01/26/2024).

Past Surgical History
has no past surgical history on file.

Meds:
[table#2]:
**Prior to Admission medications **
Medication	Sig	Start Date	End Date	Taking?	Authorizing Provider
Advair HFA 115-21 mcg/actuation inhaler	**Inhale 2 puffs 2 (two) times a day.**				**Sh Amb, Historical Provider, MD**
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	**INHALE 2 PUFFS EVERY 4 (FOUR) HOURS AS NEEDED FOR WHEEZING. CALOPTIMA MCAL ID 93931022D30310**	**10/15/24**	**11/14/24**		**Glassman, Jerrold, MD**
ammonium lactate (LAC-HYDRIN) 12 % lotion	**Apply topically 2 (two) times a day.**	**2/16/24**	**9/18/24**		**Strauser, Martha Dalia, MD**
apixaban (ELIQUIS) 5 mg tablet	**Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.**	**8/14/24**	**8/14/25**		**Glassman, Jerrold, MD**
benzonatate (TESSALON) 100 mg capsule	**Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.**	**9/23/24**	**9/30/24**		**Hernandez, Mark David, MD**
furosemide (LASIX) 40 mg tablet	**Take 1 tablet (40 mg total) by mouth daily.**	**10/17/24**	**10/17/25**		**Glassman, Jerrold, MD**
gabapentin (NEURONTIN) 300 mg capsule	**Take 2 capsules (600 mg total) by mouth 3 (three) times a day.**	**9/18/24**	**10/18/24**		**Glassman, Jerrold, MD**
metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	**TAKE 1 TABLET (25 MG TOTAL) BY MOUTH DAILY.**	**12/20/24**	**3/20/25**		**Glassman, Jerrold, MD**
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	**Take 1 tablet by mouth daily.**
Patient not taking: Reported on 9/18/2024	**2/17/24**	**9/18/24**		**Strauser, Martha Dalia, MD**
potassium chloride (MICRO-K) 10 mEq CR capsule	**Take 1 capsule (10 mEq total) by mouth daily.**	**8/14/24**	**8/14/25**		**Glassman,
Jerrold, MD**
predniSONE (DELTASONE) 20 mg tablet	**Take 2 tablets (40 mg total) by mouth daily with breakfast.**	**9/23/24**			**Hernandez, Mark David, MD**
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	**Take 1 tablet by mouth 2 (two) times a day.**	**8/14/24**	**8/14/25**		**Glassman, Jerrold, MD**

Allergies: Clindamycin and Lactose

Family History
Patient's family history includes Hypertension in her father.

Social History
Patient reports that she has quit smoking. She has been exposed to tobacco smoke. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

**Review of Systems **
Comprehensive review of systems obtained and found to be negative other than mentioned in HPI.

VItal Signs:
[table#3]:
ED Triage Vitals [12/24/24 0032]
Temp	Heart Rate	Resp	BP	SpO2
36.9 °C (98.4 °F)	**93**	**17**	**121/60**	**95 %**
Temp Source	Heart Rate Source	Patient Position	BP Location	FiO2 (%)
Oral	**--**	**--**	**--**	**--**
** **
BP 104/79 | Pulse 98 | Temp 36.9 °C (98.4 °F) (Oral) | Resp 16 | Ht 1.676 m (5' 6") | Wt 71 kg (156 lb 8.4 oz) | LMP (LMP Unknown) | SpO2 99% | BMI 25.26 kg/m²

Physical Exam.
Constitutional: well developed and nourished. non-toxic appearing.
Head: Normocephalic. Atraumatic.
Eyes: no scleral icterus. pupils round and reactive to light.
Ears: Hearing grossly intact. No traumatic lesions seen.

Nose: nose midline. No septal hematomas.
Oropharynx: Oropharynx is clear. No tonsillar hypertrophy. No bulging or erythema of the floor of the mouth or soft palate.
**Neck: **No midline bony step-off or crepitus. Range of motion intact.
Respiratory: no respiratory distress. CTAB. no wheezes or rales. Speaking in full sentences. Equal chest rise. No accessory muscle use. No crackles at the bases.
Cardiovascular: normal rate. regular rhythm. no murmur. no cyanosis
Gastrointestinal: Abdomen is soft. Nondistended. Normoactive bowel sounds. No point area of tenderness.
Musculoskeletal: ranging neck normally. no traumatic lesions.
GU: deferred
Neurologic: Patient is awake and alert. No focal or lateralizing findings. Cranial nerves II through XII are grossly intact. No facial droop or asymmetry.
Skin: no jaundice or pallor. no petechia or purpura. No cyanosis
Psychiatric: calm. normal affect.

**Labs (all labs /ekg/ reviewed and interpreted by myself unless stated otherwise **
[table#4]:
Recent Results (from the past 24 hours)
Basic metabolic panel
	Collection Time: 12/24/24 12:56 AM
Result	Value	Ref Range
	Fasting?		
	Glucose	91	70 - 125 mg/dL
	Sodium	141	137 - 145 mmol/L
	Potassium	4.4	3.5 - 5.1 mmol/L
	Chloride	103	98 - 107 mmol/L
	CO2, Total	30	22 - 30 mmol/L
	Urea Nitrogen	36 (H)	7 - 17 mg/dL
	Creatinine	1.25 (H)	0.50 - 1.00 mg/dL
	CKD-EPI eGFR 2021	49 (L)	>60 mL/min/1.73m*2
	Anion Gap	8	6 - 14 mmol/L
	Osmolality Calculated	300	280 - 305 mOsm/kg H20
	Calcium	9.3	8.4 - 10.2 mg/dL
Troponin I
	Collection Time: 12/24/24 12:56 AM
Result	Value	Ref Range
	Troponin I	0.041 (H)	<=0.034 ng/mL
NT Pro-BNP
	Collection Time: 12/24/24 12:56 AM
Result	Value	Ref Range
	NT pro-BNP	1,560 (H)	Refer to the reference range comment. pg/mL
CBC with Differential
	Collection Time: 12/24/24 12:56 AM
Result	Value	Ref Range
	WBC	6.4	3.4 - 11.0 K/mcL
	RBC	3.16 (L)	3.98 - 5.25 M/mcL
	HGB	9.5 (L)	11.9 - 15.3 g/dL
	HCT	31.7 (L)
37.3 - 46.7 %
	MCV	100	81 - 100 fL
	MCH	30	26 - 33 pg
	MCHC	30.0 (L)	31.0 - 36.0 g/dL
	RDW	14.6	11.7 - 14.9 %
	Platelet Count	411	150 - 425 K/mcL
	MPV	8.1 (L)	9.0 - 12.8 fL
	nRBC	0.0	0.0 /100 WBCs
	Neutrophils %	52.3	38.0 - 74.0 %
	IG%	4.1 (H)	0.0 - 1.2 %
	Lymphocytes %	31.8	16.0 - 48.0 %
	Monocytes %	6.3	4.9 - 12.5 %
	Eosinophils %	4.9	0.0 - 9.5 %
	Basophils %	0.6	0.0 - 1.6 %
	Absolute Neutrophils	3.32	1.50 - 7.40 K/mcL
	IG#	0.26 (H)	0.00 - 0.10 K/mcL
	Absolute Lymphocytes	2.02	0.90 - 3.10 K/mcL
	Absolute Monocytes	0.40	0.26 - 0.87 K/mcL
	Absolute Eosinophils	0.31	0.00 - 0.51 K/mcL
	Absolute Basophils	0.04	0.00 - 0.09 K/mcL
	Preliminary Abs Neut	3.32	1.50 - 7.40 K/mcL
Troponin I

	Collection Time: 12/24/24 2:46 AM
Result	Value	Ref Range
	Troponin I	0.053 (H)	<=0.034 ng/mL

**Imaging:(all imaging studies reviewed and interpreted by myself unless stated otherwise **
[table#5]:
X-ray portable chest single frontal view (Results Pending)

** ED COURSE:**

Patient's workup returned unremarkable. She has a multitude of nonspecific changes but they are all stable compared to prior. Patient has remained chest pain-free here in the emergency department. Her main concern was this midodrine. Her instructions were to hold it if it was 130 and she did not understand the meaning of that. Medication instructions were explained to the patient. She is encouraged on follow-up with her primary physician. Return precautions were provided.

The patient's presentation and medical history required multiple tests with moderate to complex medical decision making in order to determine that the patient appears safe for discharge.

[table#6]:
ED Course as of 12/24/24 0346
Tue Dec 24, 2024
0053	**12-Lead ECG**
Interpreted by me and shows sinus rhythm at a rate of 86. PR interval 178. QT corrected 490. Prolonged QT. No acute ST elevations. [DB]
0224	**NT pro-BNP(!): 1,560** [DB]
0224	**Troponin I(!): 0.041** [DB]
0226	**Creatinine(!): 1.25**
Improved compared to recent tests. [DB]
0226	**Troponin I(!): 0.041**
Previous test from New Jersey where high-sensitivity troponin so cannot be correlated. Those had been elevated as well relative to their scale. [DB]
0226	**NT pro-BNP(!): 1,560**
Stable compared to the previous values. [DB]
0341	**HGB(!): 9.5**
Stable compared to prior studies [DB]
0342	Delta troponin went from 0.04-0.05 which is essentially unchanged. Patient continues to be chest pain-free. [DB]
ED Course User Index
[DB] Ballesteros, Douglas Anthony, MD

Cardiac monitoring ordered and shows sinus rhythm without ectopy

External Notes Reviewed: Discharge summary from December 20. These notes have assisted with differential diagnosis formulation and management.

**Independent Historian: **EMS

Differential Diagnosis: Hypotension. Atrial fibrillation. Bradycardia.

Patient's care may be negatively impacted by access to care. Based on these, patient will have barriers to primary care follow-up.

ED Meds:
Medications - No data to display

Procedures

Clinical Impression:
[table#7]:
1.	**Hypotension, unspecified hypotension type Acute**

Follow-up:
Primary physician

Schedule an appointment as soon as possible for a visit in 3 days
For re-check

Disposition:

Home in stable condition

Medications and Referrals:
[table#8]:
ED Prescriptions
None
[table#9]:

Part of this note was completed using Nuance dictation system which could lead to errors during speech to text transcription.

Ballesteros, Douglas Anthony, MD
12/24/24 0346

Electronically signed by Douglas Ballesteros, MD at 12/24/2024 3:46 AM PST

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
Pneumococcal 50+ (1 of 2 - PCV)	**07/29/1983**		
Breast Cancer Screening	**07/29/2004**		
Colorectal Cancer Screening	**07/29/2009**		
RSV (1 - Risk 50-74 years 1-dose series)	**07/29/2014**		
Shingrix Vaccine (1 of 2)	**07/29/2014**		
Cervical Cancer Screening	**02/27/2024**	02/27/2019, 01/20/2012	
Flu Shot (#1)	**09/01/2026**	11/20/2012	
Adult Tdap/Td Vaccines (2 - Td or Tdap)	02/09/2034	02/09/2024	

Results - documented in this encounter

TROPONIN I - Final result (12/24/2024 2:46 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.053	<=0.034 ng/mL		12/24/2024 3:20 AM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population.  This patient's result is suspicious for myocardial injury. However,
serial measurements may be necessary to confirm or exclude the diagnosis of acute myocardial infarction.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 2:46 AM PST	12/24/2024 2:50 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (12/24/2024 1:44 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/24/2024 8:10 AM PST	

Impressions

12/24/2024 8:50 AM PST
IMPRESSION: Similar exam with no acute cardiac or pulmonary disease appreciated

release

Narrative

12/24/2024 8:50 AM PST

Narrative

```
EXAMINATION:  X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME:  12/24/2024 1:44 AM

HISTORY:  chest pain

COMPARISON: 9/23/2024

FINDINGS:

LUNGS:  Normal. No infiltrates or other abnormalities.
VASCULATURE:  Normal. Unremarkable pulmonary vasculature.
CARDIAC:  Normal. No cardiac silhouette abnormality.
MEDIASTINUM:  Normal. No visible mass or adenopathy.
PLEURA:  Normal. No effusion or pleural thickening.
BONES:  Normal. No fracture or visible bony lesion.
OTHER:  Negative.
```

Procedure Note

Trevor Nelson, MD - 12/24/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/24/2024 1:44 AM

HISTORY: chest pain

COMPARISON: 9/23/2024

FINDINGS:

LUNGS: Normal. No infiltrates or other abnormalities.
VASCULATURE: Normal. Unremarkable pulmonary vasculature.
CARDIAC: Normal. No cardiac silhouette abnormality.
MEDIASTINUM: Normal. No visible mass or adenopathy.
PLEURA: Normal. No effusion or pleural thickening.
BONES: Normal. No fracture or visible bony lesion.
OTHER: Negative.

IMPRESSION:
IMPRESSION: Similar exam with no acute cardiac or pulmonary disease appreciated

release

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	6.4	3.4 - 11.0 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
RBC	3.16	3.98 - 5.25 M/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
HGB	9.5	11.9 - 15.3 g/dL		12/24/2024 1:05 AM PST	MERCY LAB CV	
HCT	31.7	37.3 - 46.7 %		12/24/2024 1:05 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCV	100	81 - 100 fL		12/24/2024 1:05 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		12/24/2024 1:05 AM PST	MERCY LAB CV	
MCHC	30.0	31.0 - 36.0 g/dL		12/24/2024 1:05 AM PST	MERCY LAB CV	
RDW	14.6	11.7 - 14.9 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Platelet Count	411	150 - 425 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
MPV	8.1	9.0 - 12.8 fL		12/24/2024 1:05 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		12/24/2024 1:05 AM PST	MERCY LAB CV	
Neutrophils %	52.3	38.0 - 74.0 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
IG%	4.1	0.0 - 1.2 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Lymphocytes %	31.8	16.0 - 48.0 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Monocytes %	6.3	4.9 - 12.5 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Eosinophils %	4.9	0.0 - 9.5 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Basophils %	0.6	0.0 - 1.6 %		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.32	1.50 - 7.40 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
IG#	0.26	0.00 - 0.10 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.02	0.90 - 3.10 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Monocytes	0.40	0.26 - 0.87 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.31	0.00 - 0.51 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	
Preliminary Abs Neut	3.32	1.50 - 7.40 K/mcL		12/24/2024 1:05 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	1,560	Refer to the reference range comment. pg/mL		12/24/2024 1:31 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

NT-proBNP reference range: <=125 pg/mL

NT-proBNP testing aids in the diagnosis or exclusion of heart failure (HF) when used in conjunction with clinical history, symptoms, exam findings, and additional imaging and laboratory studies.

Emergency Department:
The ICON-RELOADED study provided validation of cutoff values for NT-proBNP in patients presenting with dyspnea in the Emergency Department (1).
In this setting NT-proBNP level <300 pg/mL has a high negative predictive value for HF. For values above 300 pg/mL, age-specific cutoffs provide additional decision limit categories:

Result.....Age.....Interpretation
<300.....All.....Negative: Heart Failure Unlikely
=300 to <450.....22-<50..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <900.....50-<75..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <1800....>=75....Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=>450.....22-<50..Positive: Heart Failure Likely
=>900.....50-<75..Positive: Heart Failure Likely
=>1800.....>=75....Positive: Heart Failure Likely

* Natriuretic peptides values in the gray zones may be caused by conditions other than heart failure. Acute coronary syndrome,pulmonary embolism,pulmonary hypertension,sepsis,stroke,and renal dysfunction will elevate NT- proBNP levels. Obesity, flash pulmonary edema,cardiac tamponade,and pericardial constriction are conditions associated with reduced NT-proBNP(2).

Ambulatory Clinics:
For patients presenting to outpatient clinics with suspicion of HF, the optimal clinical utility of NT-proBNP is to
exclude heart failure.For this clinical setting Ortho Clinical Diagnostics recommends NT-proBNP results be interpreted
as in the table below(2):

Result.....Age.....Interpretation
<125.....All.....Negative: Heart Failure Unlikely
>=125.....All.....Consider Heart Failure as well as other causes* of NT-proBNP elevation.

* Natriuretic peptides value elevations could also be caused by conditions other than heart failure such as acute
coronary syndrome, pulmonary embolism, pulmonary hypertension, sepsis, stroke, and renal dysfunction (2).

References:
1. Terminal Pro-B-Type Natriuretic Peptide in the Emergency Department: The ICON-RELOADED Study. J Am Coll Cardiol. 2018 Mar 20;71(11):1191-1200. doi: 10.1016/j.jacc.2018.01.021. PMID: 29544601.

2. NBNP2 Instructions For Use. Ortho Clinical Diagnostics

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.041	<=0.034 ng/mL		12/24/2024 1:31 AM PST	MERCY LAB CV	

Comment:

The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit in a healthy reference population. This patient's result is suspicious for myocardial injury. However, serial measurements may be necessary to confirm or exclude the diagnosis of acute myocardial infarction.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (12/24/2024 12:56 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/24/2024 1:18 AM PST	MERCY LAB CV	
Glucose	91	70 - 125 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	
Sodium	141	137 - 145 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	
Urea Nitrogen	36	7 - 17 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	
Creatinine	1.25	0.50 - 1.00 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	49	>60 mL/min/1.73m*2		12/24/2024 1:18 AM PST	MERCY LAB CV	
Anion Gap	8	6 - 14 mmol/L		12/24/2024 1:18 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality Calculated	300	280 - 305 mOsm/kg H20		12/24/2024 1:18 AM PST	MERCY LAB CV	
Calcium	9.3	8.4 - 10.2 mg/dL		12/24/2024 1:18 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/24/2024 12:56 AM PST	12/24/2024 12:58 AM PST

Narrative

MERCY LAB CV - 12/24/2024 1:18 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (12/24/2024 12:50 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	86	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	86	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	178	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	100	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	410	ms			SCRIPPS RADIOLOGY	
QTC	490	ms			SCRIPPS RADIOLOGY	
P AXIS	59	degrees			SCRIPPS RADIOLOGY	
R AXIS	16	degrees			SCRIPPS RADIOLOGY	
T AXIS	91	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/24/2024 12:50 AM PST	12/24/2024 10:12 AM PST

Impressions

SCRIPPS RADIOLOGY - 12/24/2024 10:12 AM PST

Impressions

Normal sinus rhythm with sinus arrhythmia
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific ST and T wave abnormality
Prolonged QT
Abnormal ECG
When compared with ECG of 23-Sep-2024 10:49,
aberrant conduction is no longer present

Narrative

This result has an attachment that is not available.

Procedure Note

Douglas Ballesteros, MD - 12/24/2024

Formatting of this note might be different from the original.

IMPRESSION:
Normal sinus rhythm with sinus arrhythmia
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific ST and T wave abnormality
Prolonged QT
Abnormal ECG
When compared with ECG of 23-Sep-2024 10:49,
aberrant conduction is no longer present

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

Visit Diagnoses - documented in this encounter

Diagnosis
Hypotension, unspecified hypotension type - Primary

Insurance - documented as of this encounter

Guarantor: Petruccio, Elizabeth Tina

Account Type	Relation to Patient	Date of Birth	Phone	Billing Address
Personal/Family	Self	07/29/1964		HOMELESS SAN DIEGO, CA 99999

BLUE SHIELD PROM MEDI-CAL

Member	Subscriber	Plan / Payer (Effective 03/01/2024-04/30/2025)
Name: Petruccio, Elizabeth Tina Member ID: AJC913100970 Relation to Subscriber: Self	Name: Petruccio, Elizabeth Tina Subscriber ID: AJC913100970	Payer ID: 671 (NAIC) Group ID: E0001001 Type: Medi-Cal HMO Phone: 800-468-9935 Address: PO BOX 272660 CHICO, CA 95927-2660

Advance Directives

For more information, please contact: 858-927-7132 (6AM - 4PM America/Los_Angeles, Monday-Friday)

Full Code (Latest Code Status on File)

Date Activated	Date Inactivated	Comments
1/12/2024 9:06 PM	2/17/2024 6:01 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:20 PM	12/23/2021 8:29 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:09 PM	12/22/2021 10:20 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers

Document Coverage Dates

Dec. 24, 2024

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

Encounter Providers

Encounter Date

Douglas Ballesteros, MD (Attending, Admitting)
NPI: 1275572372
619-691-7290 (Work)
619-691-7435 (Fax)
435 H St
Chula Vista, CA 91910-4307
Emergency Medicine

Dec. 24, 2024

Elizabeth Tina Petruccio

Summary of Care, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
10/30/2024 2:30 PM PDT	Telemedicine	San Diego Cardiac Care 4060 4th Ave Suite 650 SAN DIEGO, CA 92103-2118 858-900-9778	**Jerrold Glassman, MD** 4060 Fourth Ave #650 San Diego, CA 92103 619-819-7222 (Work) 619-299-5023 (Fax)	

Allergies - documented as of this encounter (statuses as of 10/13/2026)

Active Allergy	Reactions	Criticality	Noted Date	Comments
Clindamycin	Other (see comments)		09/23/2024	Patient unsure of what reaction she has from medication.
Lactose	Other (see comments)	Low	01/20/2024	Patient having diarrhea; only fluid milk - all other lactose items ok

Inactive Allergy	Reactions	Criticality	Noted Date	Inactivity Date	Comments
Lincosamides	Other (see comments), Unknown Reaction		07/11/2012	09/23/2024	

Medications - documented as of this encounter (statuses as of 10/13/2026)

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.						Active
ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.				02/16/2024		Active
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			02/17/2024		Active
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3		08/14/2024		Active
gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule			09/18/2024		Active
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule			09/23/2024		Active
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024		Active
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/ actuation inhaler	INHALE 2 PUFFS EVERY 4 (FOUR) HOURS AS NEEDED FOR WHEEZING. CALOPTIMA MCAL ID 93931022D30310	90 g	3		10/15/2024		Active
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3		08/14/2024	11/02/2025	Discontinued
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3		08/14/2024	11/02/2025	Discontinued
metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	Take 1 tablet (25 mg total) by mouth daily.	90 tablet			09/18/2024	12/20/2024	Discontinued
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3		10/17/2024	11/03/2025	Discontinued

Active Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date
Gallstones	02/18/2024	
Gall bladder pain	02/18/2024	
Pus collection in space between lung and chest wall	01/26/2024	
Acute osteomyelitis of cervical spine	01/26/2024	
Heart failure	01/13/2024	
Loculated pleural effusion	01/13/2024	
Atrial fibrillation (irregular heartbeat)	01/13/2024	
Respiratory insufficiency	01/12/2024	
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Resolved Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date	Resolved Date
Severe sepsis	02/18/2024		03/24/2024

Immunizations - documented as of this encounter

Immunization	Administration Dates	Next Due
Tdap greater than or equal to 7yrs IM	02/09/2024	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Passive Smoke Exposure: Past				
Smokeless Tobacco: Never				

Patient History	Answer	Date Recorded
Alcohol Use Status	Not Currently	04/18/2024
Oz/Week (Alcohol Misuse Male >= 9 OR Female >= 4.8)	Not on file	04/18/2024

Housing Stability	Answer	Date Recorded
What is your living situation today?	Do not have a stable place to live	01/19/2024

Food Insecurity	Answer	Date Recorded
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024

Transportation Needs	Answer	Date Recorded
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024

Pregnant	Comments
No	

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

Last Filed Vital Signs - documented in this encounter

Not on file

Progress Notes - documented in this encounter

Jerrold Glassman, MD - 10/30/2024 2:30 PM PDT

Formatting of this note might be different from the original.

Telephonic CARDIOLOGY PROGRESS NOTE
PCP: No primary care provider on file.
REFERRING PROVIDER: No ref. provider found
CHIEF COMPLAINT / REASON FOR VISIT : Hospital follow-up

HISTORY OF PRESENT ILLNESS:
Elizabeth Tina Petruccio is a very pleasant 60 y.o. female with new onset heart failure, was seen at Mercy Hospital and started on medications for heart failure with reduced ejection fraction. She is now on guided medical therapy. She has been asked to be compliant with her medications and visit follow-up with me. Her EF was 20%, and if remains low we will need to consider ourselves with possible ICD. She is taking Eliquis, Lasix, Entresto, as well as inhaler.

PAST MEDICAL HISTORY:
No diagnosis found.

[table#1]:
Current Outpatient Medications
Medication	Sig	Dispense	Refill
•	Advair HFA 115-21 mcg/actuation inhaler	Inhale 2 puffs 2 (two) times a day.		
•	albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	INHALE 2 PUFFS EVERY 4 (FOUR) HOURS AS NEEDED FOR WHEEZING. CALOPTIMA MCAL ID 93931022D30310	90 g	3
•	ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.		
•	apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3
•	benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule	0

•	furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3
•	gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule	0
•	metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	Take 1 tablet (25 mg total) by mouth daily.	90 tablet	0
•	multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily. (Patient not taking: Reported on 9/18/2024)	30 tablet	0
•	potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3
•	predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet	0
•	sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3

[table#2]:
No current facility-administered medications for this visit.

[table#3]:
Allergies
Allergen	Reactions
•	Clindamycin	Other (see comments)
		*Patient unsure of what reaction she has from medication. *
•	Lactose	Other (see comments)
		Patient having diarrhea; only fluid milk - all other lactose items ok

** Review of Systems**
12 point review of systems aside from pertinent positives covered in Subjective and Problem List, otherwise negative.

** PHYSICAL EXAM **
Labs/ Studies :

***BMP:* **
[table#4]:
Lab Results
Component	Value	Date/Time
	GLUCOSE	118	09/23/2024 11:21 AM
	CALCIUM	8.9	09/23/2024 11:21 AM
	NA	138	09/23/2024 11:21 AM
	K	4.2	09/23/2024 11:21 AM
	CO2	29	09/23/2024 11:21 AM
	CL	102	09/23/2024 11:21 AM
	BUN	23 (H)	09/23/2024 11:21 AM
	CREATININE	0.81	09/23/2024 11:21 AM

Lipids:
No results found for: "CHOL"
[table#5]:
Lab Results
Component	Value	Date/Time
	HDL	33 (L)	01/13/2024 05:14 AM

[table#6]:
Lab Results
Component	Value	Date/Time
	LDLCALC	63	01/13/2024 05:14 AM

[table#7]:
Lab Results
Component	Value	Date/Time
	TRIG	72	01/13/2024 05:14 AM

Continue present therapy,
will see in 1 to 2 months

Electronically signed by ***Jerrold Glassman, MD October 30, 2024*** 3:10 PM

Electronically signed by Jerrold Glassman, MD at 10/30/2024 3:12 PM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
Pneumococcal 50+ (1 of 2 - PCV)	**07/29/1983**		
Breast Cancer Screening	**07/29/2004**		
Colorectal Cancer Screening	**07/29/2009**		
RSV (1 - Risk 50-74 years 1-dose series)	**07/29/2014**		
Shingrix Vaccine (1 of 2)	**07/29/2014**		
Cervical Cancer Screening	**02/27/2024**	02/27/2019, 01/20/2012	
Flu Shot (#1)	**09/01/2026**	11/20/2012	
Adult Tdap/Td Vaccines (2 - Td or Tdap)	02/09/2034	02/09/2024	

Results - documented in this encounter

Not on file

Visit Diagnoses - documented in this encounter

Diagnosis
Heart failure - Primary

Advance Directives

For more information, please contact: 858-927-7132 (6AM - 4PM America/Los_Angeles, Monday-Friday)

Full Code (Latest Code Status on File)

Date Activated	Date Inactivated	Comments
1/12/2024 9:06 PM	2/17/2024 6:01 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:20 PM	12/23/2021 8:29 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:09 PM	12/22/2021 10:20 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers	Document Coverage Dates
	Oct. 30, 2024

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

Encounter Providers	Encounter Date
Jerrold Glassman, MD (Attending) NPI: 1245326883 619-819-7222 (Work) 619-299-5023 (Fax) 4060 Fourth Ave #650 San Diego, CA 92103 Cardiovascular Disease	Oct. 30, 2024

Elizabeth Tina Petruccio

Summary of Care, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Reason for Visit

Reason
Shortness of Breath

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
09/23/2024 10:26 AM PDT - 09/23/2024 4:26 PM PDT	Emergency	Scripps Mercy Hospital Chula Vista Emergency Department 435 H Street Chula Vista, CA 91910-4307 619-691-7000	**Mark Hernandez, MD** 435 H St Chula Vista, CA 91910-4307 619-691-7290 (Work) 619-691-7435 (Fax)	Discharge Disposition: Disch to Home (Routine)

Allergies - documented as of this encounter (statuses as of 10/13/2026)

Active Allergy	Reactions	Criticality	Noted Date	Comments
Clindamycin	Other (see comments)		09/23/2024	Patient unsure of what reaction she has from medication.
Lactose	Other (see comments)	Low	01/20/2024	Patient having diarrhea; only fluid milk - all other lactose items ok

Inactive Allergy	Reactions	Criticality	Noted Date	Inactivity Date	Comments
Lincosamides	Other (see comments), Unknown Reaction		07/11/2012	09/23/2024	

Medications - documented as of this encounter (statuses as of 10/13/2026)

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.						Active
ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.				02/16/2024		Active
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			02/17/2024		Active
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3		08/14/2024		Active
gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule			09/18/2024		Active
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule			09/23/2024		Active
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024		Active
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3		08/14/2024	10/16/2024	Discontinued (Reorder)
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3		08/14/2024	11/02/2025	Discontinued
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3		08/14/2024	11/02/2025	Discontinued
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/ actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g			09/18/2024	10/15/2024	Discontinued
metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	Take 1 tablet (25 mg total) by mouth daily.	90 tablet			09/18/2024	12/20/2024	Discontinued
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024	09/23/2024	Discontinued
doxycycline hyclate (VIBRAMYCIN) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 2 (two) times a day for 7 days.	14 capsule			09/23/2024	09/23/2024	Discontinued
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule			09/23/2024	09/23/2024	Discontinued
doxycycline hyclate (VIBRAMYCIN) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 2 (two) times a day for 7 days.	14 capsule			09/23/2024	09/30/2024	Expired

Active Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date
Gallstones	02/18/2024	
Gall bladder pain	02/18/2024	
Pus collection in space between lung and chest wall	01/26/2024	
Acute osteomyelitis of cervical spine	01/26/2024	
Heart failure	01/13/2024	
Loculated pleural effusion	01/13/2024	
Atrial fibrillation (irregular heartbeat)	01/13/2024	
Respiratory insufficiency	01/12/2024	
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Resolved Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date	Resolved Date
Severe sepsis	02/18/2024		03/24/2024

Immunizations - documented as of this encounter

Immunization	Administration Dates	Next Due
Tdap greater than or equal to 7yrs IM	02/09/2024	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Passive Smoke Exposure: Past				
Smokeless Tobacco: Never				

Tobacco Cessation: Counseling Given: Not Answered

Patient History	Answer	Date Recorded
Alcohol Use Status	Not Currently	04/18/2024
Oz/Week (Alcohol Misuse Male >= 9 OR Female >= 4.8)	Not on file	04/18/2024

Housing Stability	Answer	Date Recorded
What is your living situation today?	Do not have a stable place to live	01/19/2024

Food Insecurity	Answer	Date Recorded
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024

Transportation Needs	Answer	Date Recorded
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024

Pregnant	Comments
No	

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

Last Filed Vital Signs - documented in this encounter

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	111/81	09/23/2024 3:02 PM PDT	
Pulse	113	09/23/2024 2:30 PM PDT	
Temperature	37.1 °C (98.8 °F)	09/23/2024 12:35 PM PDT	
Respiratory Rate	25	09/23/2024 2:30 PM PDT	
Oxygen Saturation	97%	09/23/2024 3:02 PM PDT	
Inhaled Oxygen Concentration	-	-	
Weight	64 kg (141 lb 1.5 oz)	09/23/2024 10:11 AM PDT	
Height	167.6 cm (5' 6")	09/23/2024 10:11 AM PDT	
Body Mass Index	22.77	09/23/2024 10:11 AM PDT	

Functional Status - documented as of this encounter

Oxygen Therapy

Question	Answer	Date of Assessment	Author
Oxygen Therapy	None (Room air)	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Height and Weight

Question	Answer	Date of Assessment	Author
Weight	2257.51	09/23/2024 10:11 AM PDT	Your Scripps Health Team
Weight Method	Standing scale	09/23/2024 10:11 AM PDT	Your Scripps Health Team

BSA (Calculated - sq m)

Answer	Date of Assessment	Author
1.73	09/23/2024 10:11 AM PDT	Your Scripps Health Team

Tuberculosis Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	Denies symptoms	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Risks Present	Denies risks	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Results	TB Risk Screen Negative	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z

Pain Assessment

Question	Answer	Date of Assessment	Author
Pain Descriptors	Aching	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Frequency	Constant/ continuous	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Type	Acute pain	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Score	1	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Assessment	0-10	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Location	Generalized	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z

Morse Fall Risk

Question	Answer	Date of Assessment	Author
History of Falling	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Secondary Diagnosis	15	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Ambulatory Aids	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Intravenous Therapy/Heparin/Saline Lock	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Gait/Transferring	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Mental Status	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z

Question	Answer	Date of Assessment	Author
Morse Fall Risk Score	15	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z

Columbia Suicide Severity Rating Scale

Question	Answer	Date of Assessment	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
2. In the past month have you actually had any thoughts of killing yourself?	No	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z

Violence Risk Screening (Step 1)

Question	Answer	Date of Assessment	Author
Automatic Triggers	NA	09/23/2024 10:24 AM PDT	Nurse Sarabeth Z
Risk Factor Triggers	NA	09/23/2024 10:24 AM PDT	Nurse Sarabeth Z

Violence Risk Screening Outcome (Step 2)

Question	Answer	Date of Assessment	Author
Violence Screening Outcome	Negative Violence Risk Screen	09/23/2024 10:24 AM PDT	Nurse Sarabeth Z

Screening

Question	Answer	Date of Assessment	Author
Where was pt living / residing	Homeless, unsheltered	09/23/2024 10:25 AM PDT	Nurse Sarabeth Z

Are rigors present?

Answer	Date of Assessment	Author
No	09/23/2024 10:37 AM PDT	Nurse Sarabeth Z

Is there a suspected infection?

Answer	Date of Assessment	Author
No	09/23/2024 10:37 AM PDT	Nurse Sarabeth Z

Is the patient's mental status altered?

Answer	Date of Assessment	Author
No	09/23/2024 10:37 AM PDT	Nurse Sarabeth Z

Neurological

Question	Answer	Date of Assessment	Author
Level of Consciousness	Alert	09/23/2024 10:39 AM PDT	Your Scripps Health Team
Orientation Level	Oriented X4	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Cognition/Basic Behavior	Appropriate judgement;Follows commands;Appropriate safety awareness;Appropriate attention/ concentration;Appropriate for developmental age	09/23/2024 10:39 AM PDT	Your Scripps Health Team
Speech	Clear	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Date of Assessment	Author
Skin Color/Condition (WDL)	WDL	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Endocrine

Question	Answer	Date of Assessment	Author
Endocrine (WDL)	WDL	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Quick Updates

Question	Answer	Date of Assessment	Author
Quick Updates - Free Text	pt was able to ambulate to restroom on her own	09/23/2024 3:04 PM PDT	Your Scripps Health Team

Vitals

Question	Answer	Date of Assessment	Author
MAP (Monitor)	101	09/23/2024 12:45 PM PDT	Your Scripps Health Team

Departure Condition

Question	Answer	Date of Assessment	Author
DC DISPO & LOCATION (RN)	Private Residence	09/23/2024 4:06 PM PDT	Your Scripps Health Team

Oxygen Therapy

Question	Answer	Date of Assessment	Author
Oxygen Therapy	None (Room air)	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Tuberculosis Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	Denies symptoms	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Risks Present	Denies risks	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Results	TB Risk Screen Negative	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z

Pain Assessment

Question	Answer	Date of Assessment	Author
Pain Descriptors	Aching	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Frequency	Constant/ continuous	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Type	Acute pain	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Score	1	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Assessment	0-10	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Location	Generalized	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z

Morse Fall Risk

Question	Answer	Date of Assessment	Author
History of Falling	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Secondary Diagnosis	15	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Ambulatory Aids	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Intravenous Therapy/Heparin/Saline Lock	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Gait/Transferring	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Mental Status	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Morse Fall Risk Score	15	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z

Columbia Suicide Severity Rating Scale

Question	Answer	Date of Assessment	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
2. In the past month have you actually had any thoughts of killing yourself?	No	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z

Violence Risk Screening (Step 1)

Question	Answer	Date of Assessment	Author
Automatic Triggers	NA	09/23/2024 10:24 AM PDT	Nurse Sarabeth Z
Risk Factor Triggers	NA	09/23/2024 10:24 AM PDT	Nurse Sarabeth Z

Violence Risk Screening Outcome (Step 2)

Question	Answer	Date of Assessment	Author
Violence Screening Outcome	Negative Violence Risk Screen	09/23/2024 10:24 AM PDT	Nurse Sarabeth Z

Screening

Question	Answer	Date of Assessment	Author
Where was pt living / residing	Homeless, unsheltered	09/23/2024 10:25 AM PDT	Nurse Sarabeth Z

Are rigors present?

Answer	Date of Assessment	Author
No	09/23/2024 10:37 AM PDT	Nurse Sarabeth Z

Is there a suspected infection?

Answer	Date of Assessment	Author
No	09/23/2024 10:37 AM PDT	Nurse Sarabeth Z

Is the patient's mental status altered?

Answer	Date of Assessment	Author
No	09/23/2024 10:37 AM PDT	Nurse Sarabeth Z

Neurological

Question	Answer	Date of Assessment	Author
Level of Consciousness	Alert	09/23/2024 10:39 AM PDT	Your Scripps Health Team
Orientation Level	Oriented X4	09/23/2024 10:39 AM PDT	Your Scripps Health Team
Cognition/Basic Behavior	Appropriate judgement;Follows commands;Appropriate safety awareness;Appropriate attention/ concentration;Appropriate for developmental age	09/23/2024 10:39 AM PDT	Your Scripps Health Team
Speech	Clear	09/23/2024 10:39 AM PDT	Your Scripps Health Team

HEENT

Question	Answer	Date of Assessment	Author
HEENT (WDL)	WDL	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Endocrine

Question	Answer	Date of Assessment	Author
Endocrine (WDL)	WDL	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Vitals

Question	Answer	Date of Assessment	Author
MAP (Monitor)	101	09/23/2024 12:45 PM PDT	Your Scripps Health Team

Departure Condition

Question	Answer	Date of Assessment	Author
DC DISPO & LOCATION (RN)	Private Residence	09/23/2024 4:06 PM PDT	Your Scripps Health Team

Mental Status - documented in this encounter

Oxygen Therapy

Question	Answer	Entry Date	Author
Oxygen Therapy	None (Room air)	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Pain Assessment

Question	Answer	Entry Date	Author
Pain Descriptors	Aching	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Frequency	Constant/ continuous	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Type	Acute pain	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Score	1	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Assessment	0-10	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z
Pain Location	Generalized	09/23/2024 10:22 AM PDT	Nurse Sarabeth Z

Morse Fall Risk

Question	Answer	Entry Date	Author
History of Falling	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Secondary Diagnosis	15	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Ambulatory Aids	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Intravenous Therapy/Heparin/Saline Lock	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Gait/Transferring	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Mental Status	0	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
Morse Fall Risk Score	15	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z

Columbia Suicide Severity Rating Scale

Question	Answer	Entry Date	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z
2. In the past month have you actually had any thoughts of killing yourself?	No	09/23/2024 10:23 AM PDT	Nurse Sarabeth Z

Violence Risk Screening (Step 1)

Question	Answer	Entry Date	Author
Automatic Triggers	NA	09/23/2024 10:24 AM PDT	Nurse Sarabeth Z
Risk Factor Triggers	NA	09/23/2024 10:24 AM PDT	Nurse Sarabeth Z

Violence Risk Screening Outcome (Step 2)

Question	Answer	Entry Date	Author
Violence Screening Outcome	Negative Violence Risk Screen	09/23/2024 10:24 AM PDT	Nurse Sarabeth Z

Screening

Question	Answer	Entry Date	Author
Where was pt living / residing	Homeless, unsheltered	09/23/2024 10:25 AM PDT	Nurse Sarabeth Z

Are rigors present?

Answer	Entry Date	Author
No	09/23/2024 10:37 AM PDT	Nurse Sarabeth Z

Is there a suspected infection?

Answer	Entry Date	Author
No	09/23/2024 10:37 AM PDT	Nurse Sarabeth Z

Is the patient's mental status altered?

Answer	Entry Date	Author
No	09/23/2024 10:37 AM PDT	Nurse Sarabeth Z

Neurological

Question	Answer	Entry Date	Author
Level of Consciousness	Alert	09/23/2024 10:39 AM PDT	Your Scripps Health Team
Orientation Level	Oriented X4	09/23/2024 10:39 AM PDT	Your Scripps Health Team
Cognition/Basic Behavior	Appropriate judgement;Follows commands;Appropriate safety awareness;Appropriate attention/ concentration;Appropriate for developmental age	09/23/2024 10:39 AM PDT	Your Scripps Health Team
Speech	Clear	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Endocrine

Question	Answer	Entry Date	Author
Endocrine (WDL)	WDL	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Psychosocial

Question	Answer	Entry Date	Author
Psychosocial (WDL)	WDL	09/23/2024 10:39 AM PDT	Your Scripps Health Team

Vitals

Question	Answer	Entry Date	Author
MAP (Monitor)	101	09/23/2024 12:45 PM PDT	Your Scripps Health Team

Departure Condition

Question	Answer	Entry Date	Author
DC DISPO & LOCATION (RN)	Private Residence	09/23/2024 4:06 PM PDT	Your Scripps Health Team

Discharge Instructions - documented in this encounter

Discharge Instructions

Mark Hernandez, MD - 09/23/2024 3:31 PM PDT

Formatting of this note might be different from the original.

Evaluation today was very encouraging. We do not see distinct evidence of any acute infectious process, and at this time we kind of suspect mild decompensation of her CHF. We encourage you to aggressively maintain compliance with your medications, follow-up with your primary care doctor, any concerning return here at any time if you have any worsening or any other decline in condition.

Electronically signed by Mark Hernandez, MD at 09/23/2024 3:31 PM PDT

Electronically signed by Mark Hernandez, MD at 09/23/2024 3:37 PM PDT

Medications at Time of Discharge - documented as of this encounter

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.					
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3		08/14/2024	
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule			09/23/2024	
gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule			09/18/2024	
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024	
doxycycline hyclate (VIBRAMYCIN) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 2 (two) times a day for 7 days.	14 capsule			09/23/2024	09/30/2024
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g			09/18/2024	10/15/2024
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3		08/14/2024	10/16/2024
metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	Take 1 tablet (25 mg total) by mouth daily.	90 tablet			09/18/2024	12/20/2024
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3		08/14/2024	11/02/2025
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3		08/14/2024	11/02/2025

Ordered Prescriptions - documented in this encounter

Prescription	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024	
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule			09/23/2024	
doxycycline hyclate (VIBRAMYCIN) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 2 (two) times a day for 7 days.	14 capsule			09/23/2024	09/30/2024
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth every 8 (eight) hours for 7 days.	21 capsule			09/23/2024	09/23/2024
doxycycline hyclate (VIBRAMYCIN) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 2 (two) times a day for 7 days.	14 capsule			09/23/2024	09/23/2024
predniSONE (DELTASONE) 20 mg tablet	Take 2 tablets (40 mg total) by mouth daily with breakfast.	8 tablet			09/23/2024	09/23/2024

ED Notes - documented in this encounter

Mark Hernandez, MD - 09/23/2024 10:04 AM PDT

Formatting of this note might be different from the original.

Emergency Department Record
SCRIPPS CHULA HOSPITAL, CHULA VISTA

Date of Service: 9/23/2024
**Name: **Elizabeth Tina Petruccio
**MRN: **810760324
CSN: 10162454274
**DOB: **7/29/1964 **Age: **60 y.o.

[table#1]:
Chief Complaint
Patient presents with
•	Shortness of Breath
		*Patient comes to the ER with c/o increased shortness of breath which started about 1 week ago. Productive cough. Denies fever. Denies chest pain. States did take a 5 day course of Ampicillin at home. *

History of Present Illness:
Elizabeth Tina Petruccio is a 60 y.o. female to our emergency department with increasing shortness of breath over the last week. Patient reports productive cough with clear sputum production. She is denying fevers she is denying chills rigors night sweats, and denies any distinct areas of chest pain although she feels a distinct chest tightness sensation that been progressive over the last week. The patient has no nausea vomiting no diarrhea no palpitations no near syncope or syncope. The patient does have some chronic orthopnea which has been unchanged, and she does have a history of paroxysmal atrial fibrillation for which she is treated with Eliquis and beta-blockers. The patient is noting no travel or ill contacts.

Review of the patient's medical history notes a previous history of pulmonary edema CHF pleural effusion atrial fibrillation empyema osteomyelitis of the cervical spine gallbladder stones and a history of biliary colic.

Primary CARE provider: Indian health
Cardiology: Dr. Glassman

[table#2]:
Past Medical History:
Diagnosis	Date
•	Acute osteomyelitis of cervical spine (CMS/HCC)	01/26/2024
•	Asthma	
•	Atrial fibrillation (CMS/HCC)	
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)	01/13/2024
•	Calculus of gallbladder without cholecystitis without obstruction	02/18/2024
•	CHF (congestive heart failure) (CMS/HCC)	
•	Empyema of left pleural space (CMS/HCC)	01/26/2024

No past surgical history on file.

Medications:
[table#3]:
Current Outpatient Medications

[table#4]:
	Advair HFA 115-21 mcg/actuation inhaler
	albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler
	ammonium lactate (LAC-HYDRIN) 12 % lotion
	apixaban (ELIQUIS) 5 mg tablet
	furosemide (LASIX) 40 mg tablet
	gabapentin (NEURONTIN) 300 mg capsule
	metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet
	multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet
	potassium chloride (MICRO-K) 10 mEq CR capsule
	sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet

[table#5]:

Allergies:
[table#6]:
Allergies
Allergen	Reactions
•	Clindamycin	Other (see comments)
		*Patient unsure of what reaction she has from medication. *
•	Lactose	Other (see comments)
		Patient having diarrhea; only fluid milk - all other lactose items ok

Social History:
Elizabeth Tina Petruccio reports that she has quit smoking. She has been exposed to tobacco smoke. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Family History:
[table#7]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Review of Systems:
All systems reviewed and negative other than as noted above

Vital signs:
[table#8]:
Vitals:
	09/23/24 1011	09/23/24 1024	09/23/24 1235	09/23/24 1245
BP:	123/80			**(!) 114/93**
Pulse:	48	65	108	99
Resp:	16		19	20

Temp:	36.6 °C (97.9 °F)		37.1 °C (98.8 °F)	
TempSrc:	Oral		Oral	
SpO2:	97%		96%	93%
Weight:	64 kg (141 lb 1.5 oz)			
Height:	1.676 m (5' 6")			

**Physical exam: **

Patient is well-developed, well nourished & non-toxic appearing

HEENT: Head is atraumatic, normocephalic. Eyes are PERRL. OP is clear. Neck is supple without meningismus
Chest: Clear to auscultation bilaterally without wheezes or rhonchi

Car: Regular rate and rhythm without murmurs or rubs
Abd: Soft, non-distended, non-tender, normoactive bowel sounds
Ext: No cyanosis, clubbing or edema. No cords appreciated. No calf tenderness.
Neuro: A&O x4, Cranial nerves II through XII intact. Strength 5 out of 5 throughout. Sensory grossly intact to light touch throughout.
Skin: Good color and turgor. No petechiae.

**Labs: **
[table#9]:
Recent Results (from the past 24 hour(s))
12-Lead ECG
	Collection Time: 09/23/24 10:49 AM
Result	Value	Ref Range
	VENTRICULAR RATE EKG/MIN	101	bpm
	ATRIAL RATE	101	bpm
	PR-INTERVAL (MSEC)	158	ms
	QRS-INTERVAL (MSEC)	92	ms
	QT-INTERVAL (MSEC)	370	ms
	QTC	479	ms
	P AXIS	60	degrees
	R AXIS	14	degrees
	T AXIS	123	degrees
RSV PCR
	Collection Time: 09/23/24 11:15 AM
	Specimen: Nasopharyngeal; NP Swab
Result	Value	Ref Range
	RSV by PCR	Not Detected	Not Detected
Influenza A/B
	Collection Time: 09/23/24 11:16 AM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	Influenza A PCR	Not Detected	Not Detected
	Influenza B PCR	Not Detected	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 09/23/24 11:16 AM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected
Troponin I
	Collection Time: 09/23/24 11:21 AM
Result	Value	Ref Range
	Troponin I	<0.012	<=0.034 ng/mL
NT Pro-BNP
	Collection Time: 09/23/24 11:21 AM
Result	Value	Ref Range
	NT pro-BNP	3,460 (H)	Refer to the reference range comment. pg/mL
C-reactive protein
	Collection Time: 09/23/24 11:21 AM
Result	Value	Ref Range
	C-Reactive Protein	11.9 (H)	<10.0 mg/L
Procalcitonin
	Collection Time: 09/23/24 11:21 AM
Result	Value	Ref Range
	Procalcitonin	0.36 (H)	0.03 - 0.08 ng/mL
Activated PTT
	Collection Time: 09/23/24 11:21 AM
Result	Value	Ref Range
	aPTT	36	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds
Prothrombin time-INR
	Collection Time: 09/23/24 11:21 AM
Result	Value	Ref Range
	Protime	12.6	10.0 - 13.1 Seconds
	INR	1.1	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation
Comprehensive metabolic panel
	Collection Time: 09/23/24 11:21 AM
Result	Value	Ref Range
	Fasting?		
	Glucose	118	70 - 125 mg/dL
	Sodium	138	137 - 145 mmol/L

	Potassium	4.2	3.5 - 5.1 mmol/L
	Chloride	102	98 - 107 mmol/L
	CO2, Total	29	22 - 30 mmol/L
	Urea Nitrogen	23 (H)	7 - 17 mg/dL
	Creatinine	0.81	0.50 - 1.00 mg/dL
	CKD-EPI eGFR 2021	83	>60 mL/min/1.73m*2
	Anion Gap	7	6 - 14 mmol/L
	Osmolality Calculated	291	280 - 305 mOsm/kg H20
	Calcium	8.9	8.4 - 10.2 mg/dL
	Protein, total	7.9	6.3 - 8.2 g/dL
	Albumin	3.8	3.5 - 5.0 g/dL
	Alkaline Phosphatase	86	38 - 126 Units/L
	AST	29	14 - 36 Units/L
	ALT	15	<=34 Units/L
	Bilirubin, Tot
al	0.4	0.2 - 1.3 mg/dL
CBC with Differential
	Collection Time: 09/23/24 11:21 AM
Result	Value	Ref Range
	WBC	8.8	3.4 - 11.0 K/mcL
	RBC	3.96 (L)	3.98 - 5.25 M/mcL
	HGB	11.5 (L)	11.9 - 15.3 g/dL
	HCT	37.7	37.3 - 46.7 %
	MCV	95	81 - 100 fL
	MCH	29	26 - 33 pg
	MCHC	30.5 (L)	31.0 - 36.0 g/dL
	RDW	14.4	11.7 - 14.9 %
	Platelet Count	336	150 - 425 K/mcL
	MPV	8.7 (L)	9.0 - 12.8 fL
	nRBC	0.0	0.0 /100 WBCs
	Neutrophils %	78.4 (H)	38.0 - 74.0 %
	IG%	0.3	0.0 - 1.2 %
	Lymphocytes %	16.9	16.0 - 48.0 %
	Monocytes %	4.2 (L)	4.9 - 12.5 %
	Eosinophils %	0.0	0.0 - 9.5 %
	Basophils %	0.2	0.0 - 1.6 %
	Absolute Neutrophils	6.89	1.50 - 7.40 K/mcL
	IG#	0.03	0.00 - 0.10 K/mcL
	Absolute Lymphocytes	1.49	0.90 - 3.10 K/mcL
	Absolute Monocytes	0.37	0.26 - 0.87 K/mcL
	Absolute Eosinophils	0.00	0.00 - 0.51 K/mcL
	Absolute Basophils	0.02	0.00 - 0.09 K/mcL
	Preliminary Abs Neut	6.89	1.50 - 7.40 K/mcL

Imaging:
x-ray chest 2 views with pa/ap and lateral

Result Date: 9/23/2024
IMPRESSION: Decreased right pleural effusion in comparison to 1/29/2024. No new infiltrate. release
[table#10]:
x-ray chest 2 views with pa/ap and lateral
Final Result
IMPRESSION: Decreased right pleural effusion in comparison to 1/29/2024. No new infiltrate.
release

ED Course & Medical Decision Making:
[table#11]:
ED Course as of 09/23/24 1527
Mon Sep 23, 2024
1132	**12-Lead ECG**
Sinus tachycardia is noted with a ventricular rate of 101. The patient has a low normal axis. She has a downsloping ST segments in 1 aVL upright T waves are noted in those leads. No ST segment elevations are noted however no other ectopy aberrancy is noted. QRS amplitude is noted to be elevated in 1 and aVL, S waves are prominent over the anterior leads with a normal R wave

progression, certainly LVH is of concern, overall stable and unchanged from previous tracings. [MH] |
| 1520 | Chest Xray negative for consolidation or pulmonary edema.

60mg IV furosemide given for diuresis, due to elevated pro-BNP concerning for worsening of CHF symptoms contributing to production cough + SOB [GA] |
| |
| **ED Course User Index**
[GA] Armijo, Gabrielle, MD
[MH] Hernandez, Mark David, MD |

Critical Care Time:
30 minutes of critical care time was spent on this patient in excess of other billable procedures. This patient's clinical presentation required my highest level of preparedness to intervene urgently and emergently. Critical care services were provided in the following manner, by chart and data review, documentation time, discussion of care with other providers, and in the ordering and management of the patient's clinical, laboratory and radiographic studies and interventions. The patient required multiple reassessments at the bedside, as well as responses to the aforementioned therapies. This case represents a high level of medical decision making and the diagnosis carries a significant morbidity and mortality.

External Notes Reviewed: Noted.

**Independent Historian: **Adequate historian.

Differential Diagnosis: This is a 60-year-old female presenting to the emergency department shortness of breath. She does have a history of COPD as well as CHF, and demonstrates bronchitic sounds are x-ray with some complaints of sputum production. Her evaluation has been very encouraging, I do not see evidence of an acute or impending emergent process but the differential is includes acute coronary syndrome myocardial infarction myocarditis endocarditis pericarditis pericardial effusion tamponade dissection pneumonia pneumothorax pn
eumomediastinum decompensated CHF with pulmonary edema pulmonary embolus to name a few. The patient's evaluation I think send tends to lend itself towards a decompensated CHF. The patient has been diuresed. She is demonstrated an ability to ambulate throughout the emergency department without oxygen requirements, and I feel that the patient can be discharged safely with instructions to continue current medications, follow-up with primary care doctor.

Patient's care may be negatively impacted by access to care. Based on these, patient will have barriers to prompt primary and specialist care follow-up.

Clinical Impression:
Acute decompensated CHF mild to moderate
Acute COPD exacerbation mild to moderate
Acute atypical pneumonia mild to moderate
Acute shortness of breath likely multifactorial moderate

Disposition:
Discharged with the plan noted above.
Discharge to follow up with primary care doctor or local clinic in 1-2 days. Strict return precautions have been reviewed. Patient leaving ambulatory and in stable condition. They appear to understand the instructions and have no questions.

Hernandez, Mark David, MD
09/23/24 1539

Electronically signed by Mark Hernandez, MD at 09/23/2024 3:39 PM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
Pneumococcal 50+ (1 of 2 - PCV)	**07/29/1983**		
Breast Cancer Screening	**07/29/2004**		
Colorectal Cancer Screening	**07/29/2009**		
RSV (1 - Risk 50-74 years 1-dose series)	**07/29/2014**		
Shingrix Vaccine (1 of 2)	**07/29/2014**		
Cervical Cancer Screening	**02/27/2024**	02/27/2019, 01/20/2012	
Flu Shot (#1)	**09/01/2026**	11/20/2012	
Adult Tdap/Td Vaccines (2 - Td or Tdap)	02/09/2034	02/09/2024	

Results - documented in this encounter

CULTURE, BLOOD (BLOOD CULTURE) - Final result (09/23/2024 4:19 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			09/28/2024 5:07 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of left upper limb / Unknown	Venipuncture / Unknown	09/23/2024 4:19 PM PDT	09/23/2024 4:21 PM PDT

Narrative

MERCY LAB CV - 09/28/2024 5:07 PM PDT
Reference range: No growth

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (09/23/2024 4:16 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			09/28/2024 5:07 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of left upper limb / Unknown	Venipuncture / Unknown	09/23/2024 4:16 PM PDT	09/23/2024 4:21 PM PDT

Narrative

MERCY LAB CV - 09/28/2024 5:07 PM PDT
Reference range: No growth

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray chest 2 views with pa/ap and lateral (X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL) - Final result (09/23/2024 11:47 AM PDT)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			09/23/2024 11:48 AM PDT	

Impressions

09/23/2024 11:49 AM PDT

Impressions

IMPRESSION: Decreased right pleural effusion in comparison to 1/29/2024. No new infiltrate.

release

Narrative

09/23/2024 11:49 AM PDT

EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 9/23/2024 11:47 AM

HISTORY: productive cough

COMPARISON: 1/29/2024

FINDINGS:

The cardiomediastinal silhouette is normal in size and contour.

Decreased right pleural effusion in comparison to 1/29/2024.

No evidence of fracture or suspicious osseous lesion.

Procedure Note

Brian Shore, MD - 09/23/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 9/23/2024 11:47 AM

HISTORY: productive cough

COMPARISON: 1/29/2024

FINDINGS:

The cardiomediastinal silhouette is normal in size and contour.

Decreased right pleural effusion in comparison to 1/29/2024.

No evidence of fracture or suspicious osseous lesion.

IMPRESSION:

IMPRESSION: Decreased right pleural effusion in comparison to 1/29/2024. No new infiltrate.

release

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	8.8	3.4 - 11.0 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
RBC	3.96	3.98 - 5.25 M/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
HGB	11.5	11.9 - 15.3 g/dL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
HCT	37.7	37.3 - 46.7 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MCV	95	81 - 100 fL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MCH	29	26 - 33 pg		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MCHC	30.5	31.0 - 36.0 g/dL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
RDW	14.4	11.7 - 14.9 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Platelet Count	336	150 - 425 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
MPV	8.7	9.0 - 12.8 fL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Neutrophils %	78.4	38.0 - 74.0 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
IG%	0.3	0.0 - 1.2 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Lymphocytes %	16.9	16.0 - 48.0 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Monocytes %	4.2	4.9 - 12.5 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Eosinophils %	0.0	0.0 - 9.5 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Neutrophils	6.89	1.50 - 7.40 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Lymphocytes	1.49	0.90 - 3.10 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Monocytes	0.37	0.26 - 0.87 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Eosinophils	0.00	0.00 - 0.51 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Absolute Basophils	0.02	0.00 - 0.09 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	
Preliminary Abs Neut	6.89	1.50 - 7.40 K/mcL		09/23/2024 11:32 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				09/23/2024 11:54 AM PDT	MERCY LAB CV	
Glucose	118	70 - 125 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Urea Nitrogen	23	7 - 17 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Creatinine	0.81	0.50 - 1.00 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
CKD-EPI eGFR 2021	83	>60 mL/min/1.73m*2		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Anion Gap	7	6 - 14 mmol/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Osmolality Calculated	291	280 - 305 mOsm/kg H20		09/23/2024 11:54 AM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Calcium	8.9	8.4 - 10.2 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Protein, total	7.9	6.3 - 8.2 g/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Alkaline Phosphatase	86	38 - 126 Units/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
AST	29	14 - 36 Units/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
ALT	15	<=34 Units/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	
Bilirubin, Total	0.4	0.2 - 1.3 mg/dL		09/23/2024 11:54 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

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MERCY LAB CV - 09/23/2024 11:54 AM PDT

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTIME-INR (PROTHROMBIN TIME-INR) - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protime	12.6	10.0 - 13.1 Seconds		09/23/2024 12:01 PM PDT	MERCY LAB CV	
INR	1.1	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:25 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

APTT (ACTIVATED PTT) - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
aPTT	36	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:25 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROCALCITONIN - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Procalcitonin	0.36	0.03 - 0.08 ng/mL		09/23/2024 12:05 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 12:05 PM PDT

Narrative
Procalcitonin (PCT) is a dynamic biomarker with value in guiding antibiotic management in select clinical settings. PCT values are most useful when trends are analyzed. Decisions on antibiotic use should not be based solely on PCT level. Consider repeating PCT no more than daily in ICU/sepsis patients, and every 2 days for LRTI in order to discontinue antibiotics. Suspected Lower Respiratory Tract Infection (LRTI): 0.1-0.25 ng/mL - low likelihood for bacterial infection, antibiotics discouraged >0.25 ng/mL - increased likelihood for bacterial infection. Suspected Sepsis 0.1-0.5 ng/mL - low likelihood for sepsis, antibiotics discouraged. >0.5 ng/mL - increased likelihood of sepsis. >2.0 ng/mL - high risk of sepsis/septic shock. Consider discontinuation of antibiotics when PCT <0.25 (LRTI) or <0.5 (sepsis), or 80% reduction from baseline.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	11.9	<10.0 mg/L		09/23/2024 11:54 AM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	3,460	Refer to the reference range comment. pg/mL		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

NT-proBNP reference range: <=125 pg/mL

NT-proBNP testing aids in the diagnosis or exclusion of heart failure (HF) when used in conjunction with clinical history, symptoms, exam findings, and additional imaging and laboratory studies.

Emergency Department:
The ICON-RELOADED study provided validation of cutoff values for NT-proBNP in patients presenting with dyspnea in the Emergency Department (1).
In this setting NT-proBNP level <300 pg/mL has a high negative predictive value for HF. For values above 300 pg/mL, age-specific cutoffs provide additional decision limit categories:

Result.....Age.....Interpretation
<300.....All.....Negative: Heart Failure Unlikely
=300 to <450.....22-<50..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <900.....50-<75..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <1800....>=75....Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=>450.....22-<50..Positive: Heart Failure Likely
=>900.....50-<75..Positive: Heart Failure Likely
=>1800.....>=75....Positive: Heart Failure Likely

* Natriuretic peptides values in the gray zones may be caused by conditions other than heart failure. Acute coronary syndrome,pulmonary embolism,pulmonary hypertension,sepsis,stroke,and renal dysfunction will elevate NT- proBNP levels. Obesity, flash pulmonary edema,cardiac tamponade,and pericardial constriction are conditions associated with reduced NT-proBNP(2).

Ambulatory Clinics:
For patients presenting to outpatient clinics with suspicion of HF, the optimal clinical utility of NT-proBNP is to
exclude heart failure.For this clinical setting Ortho Clinical Diagnostics recommends NT-proBNP results be interpreted
as in the table below(2):

Result.....Age.....Interpretation
<125.....All.....Negative: Heart Failure Unlikely
>=125.....All.....Consider Heart Failure as well as other causes* of NT-proBNP elevation.

* Natriuretic peptides value elevations could also be caused by conditions other than heart failure such as acute
coronary syndrome, pulmonary embolism, pulmonary hypertension, sepsis, stroke, and renal dysfunction (2).

References:
1. Terminal Pro-B-Type Natriuretic Peptide in the Emergency Department: The ICON-RELOADED Study. J Am Coll Cardiol. 2018 Mar 20;71(11):1191-1200. doi: 10.1016/j.jacc.2018.01.021. PMID: 29544601.

2. NBNP2 Instructions For Use. Ortho Clinical Diagnostics

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (09/23/2024 11:21 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	<0.012	<=0.034 ng/mL		09/23/2024 12:01 PM PDT	MERCY LAB CV	

Comment:
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit in a healthy reference population. This patient's result is considered normal. However, clinical correlation is suggested to determine whether serial measurements are necessary for further evaluation.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	09/23/2024 11:21 AM PDT	09/23/2024 11:24 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COVID-19, RAPID COMBO (COVID-19, RAPID (PCR/NAAT, COMBO)) - Final result (09/23/2024 11:16 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
COVID-19, Rapid (PCR/ NAAT)	Not Detected	Not Detected		09/23/2024 12:32 PM PDT	MERCY LAB CV	

Comment:
A Detected result is considered a positive result for that virus.
A positive SARS-CoV-2 result indicates that RNA from SARS-CoV-2 was detected and that the patient is considered infected and presumed contagious. A positive Influenza A or B result indicates that RNA from Influenza A or B, respectively, was detected and that the patient is considered infected and presumed contagious.

A Not Detected (negative) test result means that RNA was not present in the specimen above the limit of detection. However, it does not rule out the possibility of infection and should not be used as the sole basis for patient management decisions. Test parameters for asymptomatic patients have not been evaluated.

These results have been reported to the San Diego Dept. of Health Services as required by Title 17, California Code of regulations, Section 2500.
Test information for patients can be found at the following link: https://www.fda.gov/media/142192/download
Test information for providers can be found at the following link: https://www.fda.gov/media/142191/download

This SARS-CoV-2 Real Time RT-PCR test was performed under an Emergency Use Authorization granted by the FDA for the Roche Liat.

This test meets Covid-19 testing requirements for school and business testing criteria and is not an antibody or antigen test.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		09/23/2024 11:16 AM PDT	09/23/2024 11:23 AM PDT

Narrative

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

INFLUENZA A/B COMBO - Final result (09/23/2024 11:16 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Influenza A PCR	Not Detected	Not Detected		09/23/2024 12:32 PM PDT	MERCY LAB CV	
Influenza B PCR	Not Detected	Not Detected		09/23/2024 12:32 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		09/23/2024 11:16 AM PDT	09/23/2024 11:23 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 12:32 PM PDT
These results have been reported to the San Diego Department of Health Services as required by Title 17, California Code of Regulations, Section 2505.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RSV BY PCR (RSV (PCR/NAAT)) - Final result (09/23/2024 11:15 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
RSV by PCR	Not Detected	Not Detected		09/23/2024 12:33 PM PDT	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
NP Swab	Nasopharyngeal structure / Unknown		09/23/2024 11:15 AM PDT	09/23/2024 11:23 AM PDT

Narrative

MERCY LAB CV - 09/23/2024 12:33 PM PDT
These results have been reported to the San Diego Department of Health Services as required by Title 17, California Code of regulations, Section 2505.

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (09/23/2024 10:49 AM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	101	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	101	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	158	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	92	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	370	ms			SCRIPPS RADIOLOGY	
QTC	479	ms			SCRIPPS RADIOLOGY	
P AXIS	60	degrees			SCRIPPS RADIOLOGY	
R AXIS	14	degrees			SCRIPPS RADIOLOGY	
T AXIS	123	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			09/23/2024 10:49 AM PDT	09/23/2024 2:20 PM PDT

Impressions

```
SCRIPPS RADIOLOGY - 09/23/2024 2:20 PM PDT
Sinus tachycardia with premature atrial complexes with aberrant conduction
Minimal voltage criteria for LVH, may be normal variant ( Cornell product )
Nonspecific T wave abnormality
Abnormal ECG
When compared with ECG of 11-Feb-2024 10:50,
aberrant conduction is now present
Nonspecific T wave abnormality no longer evident in Inferior leads
T wave inversion less evident in Lateral leads
```

Narrative

This result has an attachment that is not available.

Procedure Note

Mark Hernandez, MD - 09/23/2024

Formatting of this note might be different from the original.

IMPRESSION:
Sinus tachycardia with premature atrial complexes with aberrant conduction
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Nonspecific T wave abnormality
Abnormal ECG
When compared with ECG of 11-Feb-2024 10:50,
aberrant conduction is now present
Nonspecific T wave abnormality no longer evident in Inferior leads
T wave inversion less evident in Lateral leads

Authorizing Provider	Result Type	Result Status
Mark Hernandez MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

Visit Diagnoses - documented in this encounter

Diagnosis
Heart failure - Primary
COPD exacerbation

Administered Medications - documented in this encounter

Inactive Administered Medications - up to 3 most recent administrations

Medication Order	MAR Action	Action Date	Dose	Rate	Site
benzonatate (TESSALON) capsule 100 mg 100 mg, oral, Once, On Mon 9/23/24 at 1600, For 1 dose, `Do not crush, chew, break or open. Swallow whole.	Given	09/23/2024 3:59 PM PDT	100 mg		
furosemide (LASIX) injection 60 mg 60 mg, intraVENOUS, Once, On Mon 9/23/24 at 1500, For 1 dose, `Do not refrigerate	Given	09/23/2024 2:51 PM PDT	60 mg		

Insurance - documented as of this encounter

Guarantor: Petruccio, Elizabeth Tina

Account Type	Relation to Patient	Date of Birth	Phone	Billing Address
Personal/Family	Self	07/29/1964		HOMELESS SAN DIEGO, CA 99999

BLUE SHIELD PROM MEDI-CAL

Member	Subscriber	Plan / Payer (Effective 03/01/2024-04/30/2025)
Name: Petruccio, Elizabeth Tina Member ID: AJC913100970 Relation to Subscriber: Self	Name: Petruccio, Elizabeth Tina Subscriber ID: AJC913100970	Payer ID: 671 (NAIC) Group ID: E0001001 Type: Medi-Cal HMO Phone: 800-468-9935 Address: PO BOX 272660 CHICO, CA 95927-2660

Advance Directives

For more information, please contact: 858-927-7132 (6AM - 4PM America/Los_Angeles, Monday-Friday)

Full Code (Latest Code Status on File)

Date Activated	Date Inactivated	Comments
1/12/2024 9:06 PM	2/17/2024 6:01 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:20 PM	12/23/2021 8:29 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:09 PM	12/22/2021 10:20 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers

Document Coverage Dates

Sep. 23, 2024

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

Encounter Providers

Encounter Date

Mark Hernandez, MD (Attending, Admitting)
NPI: 1952454449
619-691-7290 (Work)
619-691-7435 (Fax)
435 H St
Chula Vista, CA 91910-4307
Emergency Medicine

Sep. 23, 2024

Elizabeth Tina Petruccio

Summary of Care, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
09/18/2024 3:30 PM PDT	Office Visit	San Diego Cardiac Care 4060 4th Ave Suite 650 SAN DIEGO, CA 92103-2118 858-900-9778	**Jerrold Glassman, MD** 4060 Fourth Ave #650 San Diego, CA 92103 619-819-7222 (Work) 619-299-5023 (Fax)	

Allergies - documented as of this encounter (statuses as of 10/13/2026)

Active Allergy	Reactions	Criticality	Noted Date	Comments
Lactose	Other (see comments)	Low	01/20/2024	Patient having diarrhea; only fluid milk - all other lactose items ok

Medications - documented as of this encounter (statuses as of 10/13/2026)

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.						Active
ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.				02/16/2024		Active
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			02/17/2024		Active
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3		08/14/2024		Active
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day. Caloptima Mcal, ID 93931022D30310	60 tablet			12/23/2021	09/18/2024	Discontinued
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/ actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g	3		12/23/2021	09/18/2024	Discontinued (Reorder)
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	09/18/2024	Discontinued
dapagliflozin propanediol (FARXIGA) 10 mg tablet tablet	Take 0.5 tablets (5 mg total) by mouth daily.	45 tablet			02/17/2024	09/18/2024	Discontinued
gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule			03/11/2024	09/18/2024	Discontinued (Reorder)
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3		08/14/2024	10/16/2024	Discontinued (Reorder)
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3		08/14/2024	11/02/2025	Discontinued
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3		08/14/2024	11/02/2025	Discontinued

Active Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date
Gallstones	02/18/2024	
Gall bladder pain	02/18/2024	
Pus collection in space between lung and chest wall	01/26/2024	
Acute osteomyelitis of cervical spine	01/26/2024	
Heart failure	01/13/2024	
Loculated pleural effusion	01/13/2024	
Atrial fibrillation (irregular heartbeat)	01/13/2024	
Respiratory insufficiency	01/12/2024	
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Resolved Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date	Resolved Date
Severe sepsis	02/18/2024		03/24/2024

Immunizations - documented as of this encounter

Immunization	Administration Dates	Next Due
Tdap greater than or equal to 7yrs IM	02/09/2024	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Smokeless Tobacco: Never				

Tobacco Cessation: Counseling Given: Not Answered

Patient History	Answer	Date Recorded
Alcohol Use Status	Not Currently	04/18/2024
Oz/Week (Alcohol Misuse Male >= 9 OR Female >= 4.8)	Not on file	04/18/2024

Housing Stability	Answer	Date Recorded
What is your living situation today?	Do not have a stable place to live	01/19/2024

Food Insecurity	Answer	Date Recorded
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024

Transportation Needs	Answer	Date Recorded
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024

Pregnant	Comments
No	

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

Last Filed Vital Signs - documented in this encounter

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	100/62	09/18/2024 3:29 PM PDT	
Pulse	96	09/18/2024 3:29 PM PDT	
Temperature	37 °C (98.6 °F)	09/18/2024 3:29 PM PDT	
Respiratory Rate	16	09/18/2024 3:29 PM PDT	
Oxygen Saturation	96%	09/18/2024 3:29 PM PDT	
Inhaled Oxygen Concentration	-	-	
Weight	64.4 kg (142 lb)	09/18/2024 3:29 PM PDT	
Height	167.6 cm (5' 6")	09/18/2024 3:29 PM PDT	
Body Mass Index	22.92	09/18/2024 3:29 PM PDT	

Functional Status - documented as of this encounter

Encounter Vitals

Question	Answer	Date of Assessment	Author
Weight	2272	09/18/2024 3:29 PM PDT	Yesenia M

BSA (Calculated - sq m)

Answer	Date of Assessment	Author
1.73	09/18/2024 3:29 PM PDT	Yesenia M

Progress Notes - documented in this encounter

Jerrold Glassman, MD - 09/18/2024 3:30 PM PDT

Formatting of this note might be different from the original.

CARDIOLOGY PROGRESS NOTE
PCP: No primary care provider on file.
REFERRING PROVIDER: No ref. provider found
CHIEF COMPLAINT / REASON FOR VISIT : Hospital follow-up, heart failure decompensated

HISTORY OF PRESENT ILLNESS:
Elizabeth Tina Petruccio is a very pleasant 60 y.o. female with hospitalized patient. Patient has been noncompliant with her medications and is short of breath. Echocardiogram performed showed EF 20/%. We discussed in detail her medications and the needTo take them. She does have systolic failure. That is quite severe. Patient would benefit from close follow-up. Her living conditions are borderline.

PAST MEDICAL HISTORY:
[table#1]:
	Diagnosis	Plan
1.	**Congestive heart failure (CMS/HCC) **	
2.	Shortness of breath	
3.	History of noncompliance with medical treatment	
4.	History of pleural empyema	

[table#2]:
Current Outpatient Medications
Medication	Sig	Dispense	Refill
•	Advair HFA 115-21 mcg/actuation inhaler	Inhale 2 puffs 2 (two) times a day.		
•	ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.		
•	apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3
•	furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3
•	potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3
•	sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3
•	albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g	0
•	gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule	0
•	metoprolol succinate XL (TOPROL-XL) 25 mg 24 hr tablet	Take 1 tablet (25 mg total) by mouth daily.	90 tablet	0
•	multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily. (Patient not taking: Reported on 9/18/2024)	30 tablet	0

[table#3]:
No current facility-administered medications for this visit.

[table#4]:
Allergies
Allergen	Reactions
•	Lactose	Other (see comments)
		Patient having diarrhea; only fluid milk - all other lactose items ok

** Review of Systems**
12 point review of systems aside from pertinent positives covered in Subjective and Problem List, otherwise negative.

** PHYSICAL EXAM **

BP 100/62 (BP Location: Left arm, Patient Position: Sitting) | Pulse 96 | Temp 37 °C (98.6 °F) (Temporal) | Resp 16 | Ht 1.676 m (5' 6") | Wt 64.4 kg (142 lb) | LMP (LMP Unknown) | SpO2 96% | BMI 22.92 kg/m²

Physical Exam
[table#5]:
| RESP | Normal respiratory effort. Lungs are clear to ascultation bilaterally. |

| --- | --- |
CARDIAC	JVP is normal
CARDIAC	Auscultation: Regular rate and rhythm
CARDIAC	Normal S1, S2 S3-S4
CARDIAC	No murmurs, rubs, or gallops.

Labs/ Studies :

***BMP:* **
[table#6]:
Lab Results
Component	Value	Date/Time
	GLUCOSE	113	02/16/2024 05:45 AM
	CALCIUM	9.1	02/16/2024 05:45 AM
	NA	137	02/16/2024 05:45 AM
	K	4.4	02/16/2024 05:45 AM
	CO2	30	02/16/2024 05:45 AM
	CL	101	02/16/2024 05:45 AM
	BUN	23 (H)	02/16/2024 05:45 AM
	CREATININE	0.80	02/16/2024 05:45 AM

Lipids:
No results found for: "CHOL"
[table#7]:
| **Lab Result
s** |
| --- |
| Component | Value | Date/Time |
| | HDL | 33 (L) | 01/13/2024 05:14 AM |

[table#8]:
Lab Results
Component	Value	Date/Time
	LDLCALC	63	01/13/2024 05:14 AM

[table#9]:
Lab Results
Component	Value	Date/Time
	TRIG	72	01/13/2024 05:14 AM

Echocardiogram shows EF 20 to 25%

Assessment/Plan :

HFrEF

Medical noncompliance

***Follow Up **: *

1 month

Electronically signed by ***Jerrold Glassman, MD September 18, 2024*** 5:12 PM

Electronically signed by Jerrold Glassman, MD at 09/18/2024 5:14 PM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
Pneumococcal 50+ (1 of 2 - PCV)	**07/29/1983**		
Breast Cancer Screening	**07/29/2004**		
Colorectal Cancer Screening	**07/29/2009**		
RSV (1 - Risk 50-74 years 1-dose series)	**07/29/2014**		
Shingrix Vaccine (1 of 2)	**07/29/2014**		
Cervical Cancer Screening	**02/27/2024**	02/27/2019, 01/20/2012	
Flu Shot (#1)	**09/01/2026**	11/20/2012	
Adult Tdap/Td Vaccines (2 - Td or Tdap)	02/09/2034	02/09/2024	

Results - documented in this encounter

Not on file

Visit Diagnoses - documented in this encounter

Diagnosis
Heart failure - Primary
Shortness of breath
History of noncompliance with medical treatment
History of pleural empyema

Advance Directives

For more information, please contact: 858-927-7132 (6AM - 4PM America/Los_Angeles, Monday-Friday)

Full Code (Latest Code Status on File)

Date Activated	Date Inactivated	Comments
1/12/2024 9:06 PM	2/17/2024 6:01 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:20 PM	12/23/2021 8:29 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:09 PM	12/22/2021 10:20 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers

Document Coverage Dates

Sep. 18, 2024

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

Encounter Providers

Encounter Date

Jerrold Glassman, MD (Attending)
NPI: 1245326883
619-819-7222 (Work)
619-299-5023 (Fax)
4060 Fourth Ave #650
San Diego, CA 92103
Cardiovascular Disease

Sep. 18, 2024

Elizabeth Tina Petruccio

Summary of Care, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
08/14/2024 3:00 PM PDT	Consult	San Diego Cardiac Care 4060 4th Ave Suite 650 SAN DIEGO, CA 92103-2118 858-900-9778	**Jerrold Glassman, MD** 4060 Fourth Ave #650 San Diego, CA 92103 619-819-7222 (Work) 619-299-5023 (Fax)	

Allergies - documented as of this encounter (statuses as of 10/13/2026)

Active Allergy	Reactions	Criticality	Noted Date	Comments
Lactose	Other (see comments)	Low	01/20/2024	Patient having diarrhea; only fluid milk - all other lactose items ok

Medications - documented as of this encounter (statuses as of 10/13/2026)

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.						Active
ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.				02/16/2024		Active
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			02/17/2024		Active
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day. Caloptima Mcal, ID 93931022D30310	60 tablet			12/23/2021	09/18/2024	Discontinued
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/ actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g	3		12/23/2021	09/18/2024	Discontinued (Reorder)
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	09/18/2024	Discontinued
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 10 mEq by mouth daily.				11/04/2023	08/14/2024	Discontinued (Reorder)
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	30 tablet			02/16/2024	08/14/2024	Discontinued (Reorder)
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation.	60 tablet			02/16/2024	08/14/2024	Discontinued (Reorder)
dapagliflozin propanediol (FARXIGA) 10 mg tablet tablet	Take 0.5 tablets (5 mg total) by mouth daily.	45 tablet			02/17/2024	09/18/2024	Discontinued
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	60 tablet			02/16/2024	08/14/2024	Discontinued (Reorder)
gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule			03/11/2024	09/18/2024	Discontinued (Reorder)

Active Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date
Gallstones	02/18/2024	
Gall bladder pain	02/18/2024	
Pus collection in space between lung and chest wall	01/26/2024	
Acute osteomyelitis of cervical spine	01/26/2024	
Heart failure	01/13/2024	
Loculated pleural effusion	01/13/2024	
Atrial fibrillation (irregular heartbeat)	01/13/2024	
Respiratory insufficiency	01/12/2024	
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Resolved Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date	Resolved Date
Severe sepsis	02/18/2024		03/24/2024

Immunizations - documented as of this encounter

Immunization	Administration Dates	Next Due
Tdap greater than or equal to 7yrs IM	02/09/2024	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Smokeless Tobacco: Never				

Tobacco Cessation: Counseling Given: Not Answered

Patient History	Answer	Date Recorded
Alcohol Use Status	Not Currently	04/18/2024
Oz/Week (Alcohol Misuse Male >= 9 OR Female >= 4.8)	Not on file	04/18/2024

Housing Stability	Answer	Date Recorded
What is your living situation today?	Do not have a stable place to live	01/19/2024

Food Insecurity	Answer	Date Recorded
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024

Transportation Needs	Answer	Date Recorded
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024

Pregnant	Comments
No	

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

Last Filed Vital Signs - documented in this encounter

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	138/98	08/14/2024 2:38 PM PDT	
Pulse	90	08/14/2024 2:38 PM PDT	
Temperature	36.5 °C (97.7 °F)	08/14/2024 2:38 PM PDT	
Respiratory Rate	16	08/14/2024 2:38 PM PDT	
Oxygen Saturation	96%	08/14/2024 2:38 PM PDT	
Inhaled Oxygen Concentration	-	-	
Weight	65.3 kg (144 lb)	08/14/2024 2:38 PM PDT	
Height	167.6 cm (5' 6")	08/14/2024 2:38 PM PDT	
Body Mass Index	23.24	08/14/2024 2:38 PM PDT	

Functional Status - documented as of this encounter

Encounter Vitals

Question	Answer	Date of Assessment	Author
Weight	2304	08/14/2024 2:38 PM PDT	Your Scripps Health Team

BSA (Calculated - sq m)

Answer	Date of Assessment	Author
1.74	08/14/2024 2:38 PM PDT	Your Scripps Health Team

Progress Notes - documented in this encounter
Jerrold Glassman, MD - 08/14/2024 3:00 PM PDT
Formatting of this note might be different from the original.
CARDIOLOGY CONSULT NOTE

PCP: No primary care provider on file.
REFERRING PROVIDER: No ref. provider found
REASON FOR CONSULT / REFERRAL : Cardiomyopathy

HISTORY OF PRESENT ILLNESS:

Elizabeth Tina Petruccio is a pleasant 60 y.o. female with

History of cardiomyopathy, likely nonischemic, may be related to polysubstance drug abuse. Patient also suffered a empyema in her lung. Was hospitalized, then to a nursing home. Patient did leave AGAINST MEDICAL ADVICE, and was likely noncompliant with her medications.

We discussed her medicines, and the importance of continue to take it

PAST MEDICAL HISTORY:** **
[table#1]:
	Diagnosis	Plan
1.	**Congestive heart failure (CMS/HCC) **	
2.	Shortness of breath	
3.	History of noncompliance with medical treatment	
4.	History of pleural empyema	

SOCIAL HISTORY:
Polysubstance drug abuse

FAMILY HISTORY:
Mother:
Father:

[table#2]:
Current Outpatient Medications
Medication	Sig	Dispense	Refill
•	Advair HFA 115-21 mcg/actuation inhaler	Inhale 2 puffs 2 (two) times a day.		
•	albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g	3
•	ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.		
•	apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation, ATRIAL FIB.	180 tablet	3
•	aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet	0
•	carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day. Caloptima Mcal, ID 93931022D30310	60 tablet	0
•	dapagliflozin propanediol (FARXIGA) 10 mg tablet tablet	Take 0.5 tablets (5 mg total) by mouth daily.	45 tablet	0
•	furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	90 tablet	3
•	gabapentin (NEURONTIN) 300 mg capsule	Take 2 capsules (600 mg total) by mouth 3 (three) times a day.	180 capsule	0
•	multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet	0
•	potassium chloride (MICRO-K) 10 mEq CR capsule	Take 1 capsule (10 mEq total) by mouth daily.	90 capsule	3
•	sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	180 tablet	3

[table#3]:
No current facility-administered medications for this visit.

[table#4]:
Allergies
Allergen	Reactions
•	Lactose	Other (see comments)
		Patient having diarrhea; only fluid milk - all other lactose items ok

REVIEW OF SYSTEM
12 point review of systems aside from pertinent positives covered in HPI and Problem List/PMH, otherwise negative
History of smoking or using illicit drugs

**PHYSICAL EXAM **

BP (!) 138/98 (BP Location: Left arm, Patient Position: Sitting) | Pulse 90 | Temp 36.5 °C (97.7 °F) (Temporal) | Resp 16 | Ht 1.676 m (5' 6") | Wt 65.3 kg (144 lb) | LMP (LMP Unknown) | SpO2 96% | BMI 23.24 kg/m²

Physical Exam
[table#5]:
RESP	Normal respiratory effort. Lungs are clear to ascultation bilaterally.
CARDIAC	JVP is normal
CARDIAC	Auscultation: Regular rate and rhythm
CARDIAC	Normal S1, S2 S3-S4
CARDIAC	No murmurs, rubs, or gallops.

Labs/Studies:

BMP
[table#6]:
Lab Results
Component	Value	Date/Time
	GLUCOSE	113	02/16/2024 05:45 AM
	CALCIUM	9.1	0
2/16/2024 05:45 AM
	NA	137	02/16/2024 05:45 AM
	K	4.4	02/16/2024 05:45 AM
	CO2	30	02/16/2024 05:45 AM
	CL	101	02/16/2024 05:45 AM
	BUN	23 (H)	02/16/2024 05:45 AM
	CREATININE	0.80	02/16/2024 05:45 AM

[table#7]:
Lab Results
Component	Value	Date/Time
	WBC	5.0	02/16/2024 05:45 AM
	HGB	9.7 (L)	02/16/2024 05:45 AM
	HCT	32.3 (L)	02/16/2024 05:45 AM
	MCV	101 (H)	02/16/2024 05:45 AM
	PLT	305	02/16/2024 05:45 AM

Lipids:
No results found for: "CHOL"
[table#8]:
Lab Results
Component	Value	Date/Time
	HDL	33 (L)	01/13/2024 05:14 AM

[table#9]:
Lab Results
Component	Value	Date/Time
	LDLCALC	63	01/13/2024 05:14 AM

[table#10]:
Lab Results
Component	Value	Date/Time
	TRIG	72	01/13/2024 05:14 AM

EKG : Sinus rhythm, LVH, abnormal repolarization

Echocardiogram : Pending

Assessment/Plan :

1. Cardiomyopathy, likely nonischemic
2. Polysubstance drug abuse
3. Medical noncompliance

***Follow Up **: *

As outlined above

Electronically signed by **Jerrold Glassman, MD August 14, 2024 **4:26 PM

Electronically signed by Jerrold Glassman, MD at 08/14/2024 4:41 PM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
Pneumococcal 50+ (1 of 2 - PCV)	**07/29/1983**		
Breast Cancer Screening	**07/29/2004**		
Colorectal Cancer Screening	**07/29/2009**		
RSV (1 - Risk 50-74 years 1-dose series)	**07/29/2014**		
Shingrix Vaccine (1 of 2)	**07/29/2014**		
Cervical Cancer Screening	**02/27/2024**	02/27/2019, 01/20/2012	
Flu Shot (#1)	**09/01/2026**	11/20/2012	
Adult Tdap/Td Vaccines (2 - Td or Tdap)	02/09/2034	02/09/2024	

Results - documented in this encounter

Not on file

Visit Diagnoses - documented in this encounter

Diagnosis
Heart failure - Primary
Shortness of breath
History of noncompliance with medical treatment
History of pleural empyema

Advance Directives

For more information, please contact: 858-927-7132 (6AM - 4PM America/Los_Angeles, Monday-Friday)

Full Code (Latest Code Status on File)

Date Activated	Date Inactivated	Comments
1/12/2024 9:06 PM	2/17/2024 6:01 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:20 PM	12/23/2021 8:29 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:09 PM	12/22/2021 10:20 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers	Document Coverage Dates
	Aug. 14, 2024

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

Encounter Providers	Encounter Date
Jerrold Glassman, MD (Attending) NPI: 1245326883 619-819-7222 (Work) 619-299-5023 (Fax) 4060 Fourth Ave #650 San Diego, CA 92103 Cardiovascular Disease	Aug. 14, 2024

Elizabeth Tina Petruccio

Summary of Care, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Reason for Visit

Reason
Shortness of Breath

Inpatient Admission (Routine)

Specialty	Diagnoses / Procedures	Referred By Contact	Referred To Contact
	Diagnoses Oral yeast infection Fluid in pleural cavity Prolonged Q-T interval on ECG COPD exacerbation Acute kidney injury Respiratory insufficiency		

Referral ID	Status	Reason	Start Date	Expiration Date	Visits Requested	Visits Authorized
13393391					1	1

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
01/12/2024 7:16 PM PST - 02/17/2024 4:00 PM PST	Hospital Encounter	Scripps Mercy Hospital Chula Vista 4th FL Definitive Observation Unit 435 H Street Chula Vista, CA 91910-4307 619-691-7000	**Stephen Zanoni, MD** 435 H St Chula Vista, CA 91910-4307 619-691-7290 (Work) 619-691-7435 (Fax) **Christopher Jones, MD** 435 H Street CV 31 Chula Vista, CA 91910 619-862-6673 (Work) 619-691-7335 (Fax) **Pedro Dammert Coronado, MD** 435 H St CV 31 Chula Vista, CA 91910-4307 619-862-6673 (Work) 619-691-7335 (Fax) **Truc Ho, MD** 435 H St CV 31 Chula Vista, CA 91910-4307 619-862-6673 (Work) 619-691-7335 (Fax) **Martha Strauser, MD** 435 H St CV 31 Chula Vista, CA 91910-4307 619-862-6673 (Work) 619-691-7335 (Fax) **Diep Dao, DO** 435 H St CV 31 Chula Vista, CA 91910 619-862-6673 (Work) 619-691-7335 (Fax) **Omar Khayyat, MD** 435 H St CV 31 Chula Vista, CA 91910-4307 619-862-6673 (Work) 619-691-7335 (Fax) **Amanda de Four, MD** 435 H St CV 31 Chula Vista, CA 91910-4307 619-862-6673 (Work) 619-691-7335 (Fax)	Discharge Disposition: Disch to SNF

Allergies - documented as of this encounter (statuses as of 10/13/2026)

Active Allergy	Reactions	Criticality	Noted Date	Comments
Lactose	Other (see comments)	Low	01/20/2024	Patient having diarrhea; only fluid milk - all other lactose items ok

Medications - documented as of this encounter (statuses as of 10/13/2026)

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.						Active
ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.				02/16/2024		Active
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			02/17/2024		Active
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day. Caloptima Mcal, ID 93931022D30310	60 tablet			12/23/2021	09/18/2024	Discontinued
lisinopriL (PRINIVIL) 10 mg tablet	Take 1 tablet (10 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	02/17/2024	Discontinued (Stop Taking at Discharge)
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days. Caloptima Mcal ID 93931022D30310	21 capsule			12/23/2021	01/15/2024	Discontinued (DC EXPIRED MED)
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/ actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g	3		12/23/2021	09/18/2024	Discontinued (Reorder)
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily for 14 days. Caloptima Mcal ID 93931022D30310	14 tablet			12/23/2021	01/15/2024	Discontinued (Med Hx - Alternate Therapy)
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/23/2021	01/15/2024	Discontinued (Historical Discontinue/ Completed)
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	09/18/2024	Discontinued
furosemide (LASIX) 20 mg tablet	Take 20 mg by mouth 2 (two) times a day.				11/04/2023	01/24/2024	Discontinued (Reorder)
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 10 mEq by mouth daily.				11/04/2023	08/14/2024	Discontinued (Reorder)
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	30 tablet			02/16/2024	08/14/2024	Discontinued (Reorder)
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation.	60 tablet			02/16/2024	08/14/2024	Discontinued (Reorder)
cefTRIAXone (ROCEPHIN) 1 gram injection	Infuse 20 mL (2 g total) into a venous catheter every 24 (twenty-four) hours for 34 doses Indications: Osteomyelitis, discitis.	680 mL			02/17/2024	03/14/2024	Discontinued (Discontinued)
dapagliflozin propanediol (FARXIGA) 10 mg tablet tablet	Take 0.5 tablets (5 mg total) by mouth daily.	45 tablet			02/17/2024	09/18/2024	Discontinued
gabapentin (NEURONTIN) 100 mg capsule	Take 2 capsules (200 mg total) by mouth 3 (three) times a day.	180 capsule			02/16/2024	03/08/2024	Discontinued (Reorder)
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch	Place 2 patches on the skin every 24 (twenty-four) hours.	60 patch			02/17/2024	03/18/2024	Expired
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch	Place 1 patch on the skin every 24 (twenty-four) hours.	30 patch			02/17/2024	03/18/2024	Expired

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch	Place 2 patches on the skin every 24 (twenty-four) hours.	60 patch			02/17/2024	03/18/2024	Expired
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	60 tablet			02/16/2024	08/14/2024	Discontinued (Reorder)

Active Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date
Pus collection in space between lung and chest wall	01/26/2024	
Acute osteomyelitis of cervical spine	01/26/2024	
Heart failure	01/13/2024	
Loculated pleural effusion	01/13/2024	
Atrial fibrillation (irregular heartbeat)	01/13/2024	
Respiratory insufficiency	01/12/2024	
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Immunizations - documented as of this encounter

Immunization	Administration Dates	Next Due
Tdap greater than or equal to 7yrs IM	02/09/2024	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Smokeless Tobacco: Never				

Housing Stability	Answer	Date Recorded
What is your living situation today?	Do not have a stable place to live	01/19/2024

Food Insecurity	Answer	Date Recorded
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024

Transportation Needs	Answer	Date Recorded
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024

Pregnant	Comments
No	

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

Last Filed Vital Signs - documented in this encounter

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	125/78	02/17/2024 3:30 PM PST	
Pulse	97	02/17/2024 3:30 PM PST	
Temperature	37 °C (98.6 °F)	02/17/2024 3:30 PM PST	
Respiratory Rate	18	02/17/2024 3:30 PM PST	
Oxygen Saturation	96%	02/17/2024 3:30 PM PST	
Inhaled Oxygen Concentration	-	-	
Weight	68.8 kg (151 lb 10.8 oz)	02/17/2024 3:31 AM PST	
Height	167.6 cm (5' 6")	01/25/2024 7:00 PM PST	
Body Mass Index	24.48	01/25/2024 7:38 PM PST	

Functional Status - documented as of this encounter

Oxygen Therapy/Pulse Ox

Question	Answer	Date of Assessment	Author
O2 Flow Rate (L/min)	0	02/15/2024 11:15 AM PST	Maria G

P/F Calculation

Question	Answer	Date of Assessment	Author
FiO2 (%)	21	02/15/2024 11:15 AM PST	Maria G

O2 Delivery Method

Answer	Date of Assessment	Author
Nasal cannula	02/17/2024 7:21 AM PST	Your Scripps Health Team

Oxygen Therapy

Question	Answer	Date of Assessment	Author
Patient Activity	At rest	02/17/2024 3:30 PM PST	Your Scripps Health Team
Pulse Oximetry Type	Continuous	02/15/2024 9:20 PM PST	Your Scripps Health Team
Oxygen Therapy	None (Room air)	02/17/2024 3:30 PM PST	Your Scripps Health Team
Vent Mode	PS/CPAP	01/14/2024 10:00 AM PST	Your Scripps Health Team

Vitals

Question	Answer	Date of Assessment	Author
Weight	2426.82	02/17/2024 3:31 AM PST	Your Scripps Health Team
Weight Method	Bed Scale	02/17/2024 3:31 AM PST	Your Scripps Health Team

BSA (Calculated - sq m)

Answer	Date of Assessment	Author
1.79	02/17/2024 3:31 AM PST	Your Scripps Health Team

Provider Notification

Question	Answer	Date of Assessment	Author
Critical Lab Test Name	Other (comment)	02/04/2024 8:00 AM PST	Your Scripps Health Team
Critical Exam Test Name	Other (comment)	02/04/2024 8:00 AM PST	Your Scripps Health Team
Reason for Communication	Other (Comment)	02/15/2024 1:46 AM PST	Your Scripps Health Team
Provider Name	Jerry Chen	02/15/2024 1:46 AM PST	Your Scripps Health Team
Provider Role	Resident;Hospitalist	02/15/2024 1:46 AM PST	Your Scripps Health Team
Method of Communication	Secure chat	02/15/2024 1:46 AM PST	Your Scripps Health Team
Response	Other (Comment)	02/15/2024 1:46 AM PST	Your Scripps Health Team

Are rigors present?

Answer	Date of Assessment	Author
No	01/12/2024 7:58 PM PST	Your Scripps Health Team

Is there a suspected infection?

Answer	Date of Assessment	Author
No	01/12/2024 7:58 PM PST	Your Scripps Health Team

Is the patient's mental status altered?

Answer	Date of Assessment	Author
No	01/12/2024 7:58 PM PST	Your Scripps Health Team

BiPAP/NIV

Question	Answer	Date of Assessment	Author
IPAP (cm H2O)	14	01/13/2024 3:50 AM PST	Domingo P
FiO2 - NIV (%)	60	01/13/2024 3:20 AM PST	Domingo P
EPAP (cm H2O)	7	01/13/2024 3:50 AM PST	Domingo P
Insp Time NIV (sec)	0.8	01/13/2024 3:20 AM PST	Domingo P

Readings

Question	Answer	Date of Assessment	Author
PIP (cm H2O)	11	01/14/2024 11:11 AM PST	Ryan O
Vt Exp (mL)	340	01/14/2024 11:11 AM PST	Ryan O
Minute Ventilation (L/min)	9.5	01/14/2024 11:11 AM PST	Ryan O
MAP (cm H2O)	7	01/14/2024 11:11 AM PST	Ryan O
I:E Ratio	1:3.0	01/14/2024 7:30 AM PST	Ryan O
Static Compliance (mL/cm H2O)	34	01/14/2024 11:11 AM PST	Ryan O
Dynamic Compliance (mL/cm H2O)	22	01/13/2024 6:09 PM PST	Ji Y

Cares/ Safeties

Question	Answer	Date of Assessment	Author
Safety Assessments	Bag valve mask present;Ventilator plugged into red outlet	01/14/2024 7:30 AM PST	Ryan O

Respiratory

Question	Answer	Date of Assessment	Author
Respiratory Pattern	Normal	02/17/2024 7:15 AM PST	Your Scripps Health Team
Chest Assessment	Chest expansion symmetrical	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cough	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
R Upper Breath Sounds	Clear	02/17/2024 7:15 AM PST	Your Scripps Health Team
L Upper Breath Sounds	Clear	02/17/2024 7:15 AM PST	Your Scripps Health Team
Bilateral Breath Sounds	Clear;Diminished	02/17/2024 7:15 AM PST	Your Scripps Health Team
Dyspnea Occurrence	With exertion	02/16/2024 7:35 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
R Basilar	Diminished	02/17/2024 7:15 AM PST	Your Scripps Health Team
L Basilar	Diminished	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ventilator Patient	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Airway LDA	Endotracheal tube	01/14/2024 8:00 AM PST	Your Scripps Health Team
Respiratory Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Respiratory Effort	Unlabored	02/17/2024 7:15 AM PST	Your Scripps Health Team
Respiratory Depth/Rhythm	Regular	02/17/2024 7:15 AM PST	Your Scripps Health Team
Respiratory (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team

Tuberculosis Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	Denies symptoms	01/12/2024 8:22 PM PST	Nurse Jesse T
Risks Present	Denies risks	01/12/2024 8:22 PM PST	Nurse Jesse T
Results	TB Risk Screen Negative	01/12/2024 8:22 PM PST	Nurse Jesse T

Morse Fall Risk

Question	Answer	Date of Assessment	Author
History of Falling	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Secondary Diagnosis	15	01/12/2024 8:22 PM PST	Nurse Jesse T
Ambulatory Aids	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Intravenous Therapy/Heparin/Saline Lock	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Gait/Transferring	10	01/12/2024 8:22 PM PST	Nurse Jesse T
Mental Status	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Morse Fall Risk Score	25	01/12/2024 8:22 PM PST	Nurse Jesse T

Columbia Suicide Severity Rating Scale

Question	Answer	Date of Assessment	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	01/12/2024 8:23 PM PST	Nurse Jesse T
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	01/12/2024 8:23 PM PST	Nurse Jesse T
2. In the past month have you actually had any thoughts of killing yourself?	No	01/12/2024 8:23 PM PST	Nurse Jesse T

Violence Risk Screening (Step 1)

Question	Answer	Date of Assessment	Author
Automatic Triggers	NA	02/16/2024 7:40 AM PST	Your Scripps Health Team
Risk Factor Triggers	Substance Abuse	02/16/2024 7:40 AM PST	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Date of Assessment	Author
Violence Screening Outcome	Negative Violence Risk Screen	02/16/2024 7:40 AM PST	Your Scripps Health Team

Precaution Necessity Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Chlorhexidine Daily Bath Results	Initiate CHG bathing daily (not to be applied to open skin areas).	01/12/2024 9:29 PM PST	Nurse Jesse T
Precautions	Follow Standard Precautions	01/12/2024 9:29 PM PST	Nurse Jesse T
Patient Status	Homeless	01/12/2024 9:29 PM PST	Nurse Jesse T
Known Infection Status	None	01/12/2024 9:29 PM PST	Nurse Jesse T
Patient has an Open Draining Wound that cannot be contained by dressing	No	01/12/2024 9:29 PM PST	Nurse Jesse T

C.difficile Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	No Active Diarrhea	01/12/2024 9:29 PM PST	Nurse Jesse T
Risks Present	None of the Above	01/12/2024 9:29 PM PST	Nurse Jesse T
Results	No C. difficile test necessary	01/12/2024 9:29 PM PST	Nurse Jesse T

CRO and Candida auris Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
History of overnight hospitalization and/or invasive procedure outside of the United States within last 12 months	No	01/12/2024 9:29 PM PST	Nurse Jesse T
History of LTAC/SNF/Subacute residency within the last 12 months	No	01/12/2024 9:29 PM PST	Nurse Jesse T
(RETIRED) Transferring from any healthcare facility in Nevada state (i.e. ACH, SNF, LTAC)	No	01/12/2024 9:29 PM PST	Nurse Jesse T
Known History of CRO (Carbapenem-resistant organism)	No	01/12/2024 9:29 PM PST	Nurse Jesse T
CRO Results and Interventions	Specimen not required.	01/12/2024 9:29 PM PST	Nurse Jesse T
Known History of Candida auris	No	01/12/2024 9:29 PM PST	Nurse Jesse T
Candida auris Results and Interventions	Specimen not required.	01/12/2024 9:29 PM PST	Nurse Jesse T

Valuables Sent to Safe/Secure Area

Question	Answer	Date of Assessment	Author
Patient Valuables Sent to Safe/Secure Area	No Valuables	02/17/2024 3:23 PM PST	Your Scripps Health Team

Patient Medications

Question	Answer	Date of Assessment	Author
Medications brought by patient?	No	02/15/2024 9:30 PM PST	Your Scripps Health Team
Disposition of Medications	Sent to pharmacy	02/13/2024 8:00 PM PST	Nurse Prescilla S

Skin Color/Condition

Question	Answer	Date of Assessment	Author
Skin Color/Condition (WDL)	WDL	01/12/2024 8:23 PM PST	Nurse Jesse T

Endocrine

Question	Answer	Date of Assessment	Author
Endocrine (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team
Continuous Glucose Monitor in Use	No	02/15/2024 7:30 AM PST	Your Scripps Health Team
Needs Further Intervention for Glucose Control	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Other Active Endocrine Issue:	No	02/15/2024 7:30 AM PST	Your Scripps Health Team
(RETIRED, READ-ONLY) Rationale for Intervention Needed for Glucose Control	Other (Comment)	02/15/2024 7:30 AM PST	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	01/12/2024 8:23 PM PST	Nurse Jesse T

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	01/12/2024 8:23 PM PST	Nurse Jesse T

Vital Signs

Question	Answer	Date of Assessment	Author
EKG Rhythm	Sinus Rhythm	01/12/2024 9:34 PM PST	Nurse Jesse T

Pain Assessment

Question	Answer	Date of Assessment	Author
Pain Assessment	0-10	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Score	7	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Orientation	Right;Left	02/17/2024 2:21 AM PST	Your Scripps Health Team
Pain Interventions	Medication (See MAR);Repositioned;Rest	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Descriptors	Discomfort	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Onset	Ongoing	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Frequency	Intermittent	02/17/2024 3:48 PM PST	Your Scripps Health Team
Patient's Stated Pain Goal	No pain	02/17/2024 3:48 PM PST	Your Scripps Health Team
Response to Interventions	Effective per pt	02/17/2024 6:51 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Pain Aggravating Factors	Reposition/movement	02/17/2024 3:48 PM PST	Your Scripps Health Team
Effect of Pain on Daily Activities	Activity	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Type	Chronic pain	02/17/2024 3:48 PM PST	Your Scripps Health Team
Clinical Progression of Pain	Gradually improving	02/17/2024 2:21 AM PST	Your Scripps Health Team
VDS Pain Score	No Pain (Pain Level 0)	02/17/2024 6:51 AM PST	Your Scripps Health Team
Pain Location	Back	02/17/2024 3:48 PM PST	Your Scripps Health Team
Wong-Baker FACES Pain Rating	0	02/16/2024 4:22 PM PST	Your Scripps Health Team

Quick Updates

Question	Answer	Date of Assessment	Author
Quick Updates - Free Text	stable no distress	01/30/2024 3:49 PM PST	Your Scripps Health Team

MAP (Monitor)

Answer	Date of Assessment	Author
90	02/17/2024 7:21 AM PST	Your Scripps Health Team

Charting Type

Question	Answer	Date of Assessment	Author
Charting Type	Shift assessment	02/17/2024 7:15 AM PST	Your Scripps Health Team

Hygiene

Question	Answer	Date of Assessment	Author
Chlorhexidine Bath	Yes	02/16/2024 9:37 PM PST	Your Scripps Health Team
Oral Care	Other (Comment)	02/15/2024 2:00 PM PST	Your Scripps Health Team
Level of Assistance	Independent	02/17/2024 3:23 PM PST	Your Scripps Health Team
Hygiene	Other (Comment)	02/15/2024 2:00 PM PST	Your Scripps Health Team
Skin Care	Moisture barrier	02/10/2024 8:00 PM PST	Your Scripps Health Team
Urethral Catheter Care	Yes	02/03/2024 4:00 AM PST	Your Scripps Health Team

Sedation Scales

Question	Answer	Date of Assessment	Author
Target Arousal Level (RASS)	-2	01/13/2024 2:36 PM PST	Pedro Dammert Coronado, MD
Pasero Opioid-Induced Sedation Scale (POSS)	1	02/17/2024 3:48 PM PST	Your Scripps Health Team
Richmond Agitation Sedation Scale (RASS)	0	02/17/2024 3:48 PM PST	Your Scripps Health Team

P/F Ratio

Answer	Date of Assessment	Author
0	02/17/2024 3:07 PM PST	Background I

Feature 3: Altered Level of Consciousness

Answer	Date of Assessment	Author
Negative	02/17/2024 3:48 PM PST	Your Scripps Health Team

Pain Radiating Towards

Answer	Date of Assessment	Author
Neck	02/16/2024 3:52 PM PST	Your Scripps Health Team

Pain Alleviating Factors

Answer	Date of Assessment	Author
Meds	02/16/2024 3:52 PM PST	Your Scripps Health Team

Glasgow Coma Scale

Question	Answer	Date of Assessment	Author
Eye Opening	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Best Verbal Response	5	02/17/2024 7:15 AM PST	Your Scripps Health Team
Best Motor Response	6	02/17/2024 7:15 AM PST	Your Scripps Health Team
Glasgow Coma Scale Score	15	02/17/2024 7:15 AM PST	Your Scripps Health Team

Neurological

Question	Answer	Date of Assessment	Author
Level of Consciousness	Alert	02/17/2024 7:15 AM PST	Your Scripps Health Team
Orientation Level	Oriented X4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cognition/Basic Behavior	Appropriate judgement;Follows commands	02/17/2024 7:15 AM PST	Your Scripps Health Team
Speech	Clear	02/17/2024 7:15 AM PST	Your Scripps Health Team
Neuro Symptoms	Anxiety	02/17/2024 7:15 AM PST	Your Scripps Health Team
Neuro Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Pupil Assessment	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Motor Function/Sensation Assessment	Grip;Dorsiflexion;Plantar flexion	02/16/2024 7:40 AM PST	Your Scripps Health Team
Neuro (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team
L Pupil Reaction	Brisk	02/07/2024 8:15 AM PST	Nurse Feliccia T
L Pupil Size (mm)	2	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Pupil Reaction	Brisk	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Pupil Size (mm)	2	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Pupil Shape	Round	02/07/2024 8:15 AM PST	Nurse Feliccia T
L Pupil Shape	Round	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Hand Grip	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
L Hand Grip	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
Relieved by	Rest	02/17/2024 7:15 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
R Foot Dorsiflexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
L Foot Dorsiflexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
R Foot Plantar Flexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
L Foot Plantar Flexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
RUE Motor Strength	5-Normal muscle strength against full resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
LUE Motor Strength	5-Normal muscle strength against full resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
RLE Motor Strength	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
LLE Motor Strength	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
LUE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
LUE Sensation	Full sensation	02/16/2024 7:40 AM PST	Your Scripps Health Team
LLE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
LLE Sensation	Decreased	02/16/2024 7:40 AM PST	Your Scripps Health Team
RUE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
RUE Sensation	Full sensation	02/16/2024 7:40 AM PST	Your Scripps Health Team
RLE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
RLE Sensation	Decreased	02/16/2024 7:40 AM PST	Your Scripps Health Team

Pacemaker

Question	Answer	Date of Assessment	Author
Pacemaker	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Cardiac

Question	Answer	Date of Assessment	Author
Ectopy	Premature ventricular contractions	02/06/2024 9:00 PM PST	Your Scripps Health Team
Ectopy Frequency	Occasional	02/06/2024 9:00 PM PST	Your Scripps Health Team
Heart Sounds	S1, S2	02/16/2024 7:40 AM PST	Your Scripps Health Team
Jugular Venous Distention (JVD)	No	02/12/2024 7:50 AM PST	Your Scripps Health Team
Cardiac Symptoms	None	02/12/2024 7:50 AM PST	Your Scripps Health Team
QTc (seconds)	433	02/06/2024 8:23 AM PST	Your Scripps Health Team
Cardiac (WDL)	X	02/12/2024 7:50 AM PST	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Date of Assessment	Author
Skin Color	Appropriate for ethnicity	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin Condition/Temp	Warm;Dry	02/17/2024 7:15 AM PST	Your Scripps Health Team

Skin

Question	Answer	Date of Assessment	Author
Skin Integrity	Redness;Scaling	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin Location	both feet	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin Turgor	Non-tenting	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team

Peripheral Vascular

Question	Answer	Date of Assessment	Author
Capillary Refill	Less than/equal to 2 seconds (All extremities)	02/17/2024 7:15 AM PST	Your Scripps Health Team
Pulses	R radial;L radial;R dorsalis pedis;L dorsalis pedis	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cyanosis	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
Edema	Generalized;Right lower extremity;Left lower extremity	02/17/2024 7:15 AM PST	Your Scripps Health Team
PVS Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Peripheral Vascular (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team
Generalized Edema	Non-pitting	02/17/2024 7:15 AM PST	Your Scripps Health Team
RLE Edema	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team
LLE Edema	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team

Gastrointestinal

Question	Answer	Date of Assessment	Author
Last BM Date	66886	02/17/2024 7:15 AM PST	Your Scripps Health Team
Passing Flatus	Yes	02/17/2024 7:15 AM PST	Your Scripps Health Team
Palpation	Soft;No guarding;Nontender	02/17/2024 7:15 AM PST	Your Scripps Health Team
Bowel Sounds (All Quadrants)	Passing flatus	02/17/2024 7:15 AM PST	Your Scripps Health Team
Abdomen Inspection	Nondistended	02/17/2024 7:15 AM PST	Your Scripps Health Team
GI Symptoms	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
Gastrointestinal Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Gastrointestinal (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team

Stool Assessment

Question	Answer	Date of Assessment	Author
Bowel Incontinence	No	02/16/2024 7:40 AM PST	Your Scripps Health Team

Output (mL)

Question	Answer	Date of Assessment	Author
Stool Consistency (Bristol Stool Scale)	4-Like a sausage or snake, smooth and soft	02/10/2024 1:48 AM PST	Your Scripps Health Team
Stool Appearance	Unable to assess	02/10/2024 1:48 AM PST	Your Scripps Health Team
Stool Color	Green;Brown	02/10/2024 1:48 AM PST	Your Scripps Health Team
Urine Color	Yellow/straw	02/17/2024 11:00 AM PST	Your Scripps Health Team
Urine Appearance	Clear	02/17/2024 11:00 AM PST	Your Scripps Health Team
Urine Odor	Unable to assess	02/17/2024 11:00 AM PST	Your Scripps Health Team

Stool Output/Assessment

Question	Answer	Date of Assessment	Author
Stool Amount	Unable to assess	02/16/2024 4:00 PM PST	Your Scripps Health Team
Stool (mL)	150	02/15/2024 6:00 PM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Date of Assessment	Author
Patient Behaviors/Mood	Calm;Cooperative	02/17/2024 7:15 AM PST	Your Scripps Health Team
Needs Expressed	Physical	02/17/2024 7:15 AM PST	Your Scripps Health Team
Psychosocial Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Express Feelings	Able to express	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Express Needs	Able to express	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Express Thoughts	Able to express	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Understand Others	Understands	02/17/2024 7:15 AM PST	Your Scripps Health Team
Psychosocial (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team
Length of Time/Family Visitation	5-6 hrs	01/16/2024 8:00 AM PST	Your Scripps Health Team

HEENT

Question	Answer	Date of Assessment	Author
HEENT (WDL)	X	02/16/2024 7:35 PM PST	Your Scripps Health Team
Lips	Dry	02/14/2024 8:05 PM PST	Your Scripps Health Team

Tattoos/Piercings

Question	Answer	Date of Assessment	Author
Does patient have tattoos?	No	02/15/2024 9:30 PM PST	Your Scripps Health Team
Piercings Remaining	No	02/15/2024 9:30 PM PST	Your Scripps Health Team

Braden Scale

Question	Answer	Date of Assessment	Author
Sensory Perceptions	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Moisture	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Activity	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Mobility	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Nutrition	3	02/17/2024 7:15 AM PST	Your Scripps Health Team
Friction and Shear	3	02/17/2024 7:15 AM PST	Your Scripps Health Team
Braden Scale Score	22	02/17/2024 7:15 AM PST	Your Scripps Health Team

Anus/Rectum

Question	Answer	Date of Assessment	Author
Anus/Rectum Observations	WDL	02/16/2024 7:35 PM PST	Your Scripps Health Team

Hester Davis Fall Risk Assessment

Question	Answer	Date of Assessment	Author
Mental Status/LOC/Awareness	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Communication/Sensory	1	02/17/2024 7:15 AM PST	Your Scripps Health Team
Behavior	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Mobility	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Toileting Needs	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Patient has IV Fluids or Tube Feeds	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Last Known Fall	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
NPO Greater than 24 hours?	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Nausea/Vomiting?	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Diuretics	3	02/17/2024 7:15 AM PST	Your Scripps Health Team
Chemotherapy in the last month	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cardiac/CNS meds?	2	02/17/2024 7:15 AM PST	Your Scripps Health Team
Patient has low blood glucose or sodium	0	02/17/2024 7:15 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Hester Davis Fall Risk Score	8	02/17/2024 7:15 AM PST	Your Scripps Health Team

Urine Output/Assessment

Question	Answer	Date of Assessment	Author
Bladder Scan Volume (mL)	214	01/14/2024 5:55 AM PST	Your Scripps Health Team
Urine	500	02/17/2024 4:00 PM PST	Your Scripps Health Team
Urinary Incontinence	No	02/17/2024 4:00 PM PST	Your Scripps Health Team
Intermittent/Straight Cath (mL)	600	01/13/2024 11:35 PM PST	Your Scripps Health Team
Post Void Residual (mL)	0	01/13/2024 11:40 PM PST	Your Scripps Health Team
Urine Amount	Unable to assess	02/16/2024 4:00 AM PST	Your Scripps Health Team
Unmeasured Urine Occurrence	1	02/16/2024 4:00 AM PST	Your Scripps Health Team

Unmeasured Output

Question	Answer	Date of Assessment	Author
Unmeasured Stool Occurrence	1	02/17/2024 11:00 AM PST	Your Scripps Health Team

Precautions

Question	Answer	Date of Assessment	Author
Sacral Dressing	Applied	02/16/2024 6:00 AM PST	Your Scripps Health Team
Precautions	Fall risk	02/17/2024 3:23 PM PST	Your Scripps Health Team

Safe Environment

Question	Answer	Date of Assessment	Author
Safe Environment	Reviewed & Completed	02/17/2024 3:23 PM PST	Your Scripps Health Team

Assistive Devices at Bedside

Question	Answer	Date of Assessment	Author
Patient Assistive Devices at Bedside	Dentures;Vision	02/17/2024 3:23 PM PST	Your Scripps Health Team
Dentures	Uppers	02/17/2024 3:23 PM PST	Your Scripps Health Team
Vision - Corrective Lenses	Glasses	02/17/2024 3:23 PM PST	Your Scripps Health Team
Comment Assistive Devices at Beds	cell phone at bedside	02/16/2024 8:50 AM PST	Your Scripps Health Team

Family/Significant Other Communication

Question	Answer	Date of Assessment	Author
Did you engage the family?	No Family Identified	02/13/2024 8:00 PM PST	Nurse Prescilla S
How was family/ significant other updated?	Face to Face	01/16/2024 8:00 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
How did you engage the family?	Update on Patient Status	01/16/2024 8:00 PM PST	Your Scripps Health Team

Vent Settings and Readings

Question	Answer	Date of Assessment	Author
Resp Rate (Set)	20	01/14/2024 8:00 AM PST	Your Scripps Health Team
Vt (Set, mL)	400	01/14/2024 8:00 AM PST	Your Scripps Health Team

Settings

Question	Answer	Date of Assessment	Author
PEEP/CPAP (cm H2O)	5	01/14/2024 11:11 AM PST	Ryan O
Insp Time (sec)	0.74	01/14/2024 7:30 AM PST	Ryan O

Vital Signs

Question	Answer	Date of Assessment	Author
ETCO2 (Monitor)	44	01/26/2024 10:00 PM PST	Your Scripps Health Team

Cough/Sputum Description

Question	Answer	Date of Assessment	Author
Sputum Amount	None	02/15/2024 9:30 PM PST	Your Scripps Health Team
Sputum Color	None	02/11/2024 8:30 PM PST	Your Scripps Health Team
Sputum Consistency	None	02/07/2024 8:15 AM PST	Nurse Feliccia T
Sputum How Obtained	Spontaneous cough	01/23/2024 8:30 PM PST	Your Scripps Health Team
Cough Mechanism	Spontaneous	01/22/2024 7:40 AM PST	Your Scripps Health Team

Procedure Drainage Output

Question	Answer	Date of Assessment	Author
Misc Procedure Drainage Output (mL)	300	01/25/2024 11:00 AM PST	Your Scripps Health Team

Critical-Care Pain Observation Tool

Question	Answer	Date of Assessment	Author
Facial Expression	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Body Movements	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Compliance with the Ventilator (Intubated Patients)	0	01/14/2024 10:00 AM PST	Your Scripps Health Team
Vocalization (Extubated Patients)	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Muscle Tension	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Critical-Care Pain Observation Score	0	01/14/2024 12:00 PM PST	Your Scripps Health Team

Assistive Devices Sent Home

Question	Answer	Date of Assessment	Author
Patient Assistive Devices Sent Home	None	02/15/2024 9:30 PM PST	Your Scripps Health Team

Fall Risk Interventions

Question	Answer	Date of Assessment	Author
Fall signage in place	Yes	02/17/2024 3:23 PM PST	Your Scripps Health Team
Wristband fall alert on	Yes	02/17/2024 3:23 PM PST	Your Scripps Health Team
Room Door Open	Yes	02/17/2024 3:23 PM PST	Your Scripps Health Team
Gait Belt Used	Not applicable	02/17/2024 3:23 PM PST	Your Scripps Health Team
Alarm On	Bed	02/17/2024 3:23 PM PST	Your Scripps Health Team
Other fall interventions	frerquent rounding	02/17/2024 3:23 PM PST	Your Scripps Health Team

Mobility

Question	Answer	Date of Assessment	Author
Mobility/Exercise Safety Screen	Proceed with mobilization - No exclusion criteria met	02/10/2024 9:23 AM PST	Your Scripps Health Team
Level of Assistance	Independent	02/17/2024 3:23 PM PST	Your Scripps Health Team
Mode of Transport	Ambulatory	02/17/2024 3:23 PM PST	Your Scripps Health Team
Repositioned	Turns self	02/17/2024 3:23 PM PST	Your Scripps Health Team
Range of Motion - Passive	All extremities	02/06/2024 8:23 AM PST	Your Scripps Health Team
Head of Bed Elevated	Self regulated	02/17/2024 12:00 AM PST	Your Scripps Health Team
Anti-Embolism Intervention (Intermittent Pneumatic Compression Stockings)	Refused	02/17/2024 7:15 AM PST	Your Scripps Health Team
Heels/Feet	Heels elevated off bed	02/17/2024 2:00 PM PST	Your Scripps Health Team
Anti-Embolism Devices	Sequential compression devices, below knee	02/17/2024 7:15 AM PST	Your Scripps Health Team
Anti-Embolism Device Location	Bilateral lower extremities	02/17/2024 7:15 AM PST	Your Scripps Health Team
Range of Motion - Active	All extremities	02/17/2024 12:00 AM PST	Your Scripps Health Team
$ Positioning Device Supply Charges	Boot Truvue (Heel Protectors)	02/07/2024 8:00 AM PST	Nurse Feliccia T
Assistive Device	None	02/14/2024 10:50 PM PST	Your Scripps Health Team
Mobility Level (Calculated)	Level 4	02/17/2024 3:23 PM PST	Your Scripps Health Team
Mobilization Response	Tolerated well	02/17/2024 12:00 AM PST	Your Scripps Health Team
Distance Ambulated (ft)	12	02/16/2024 7:35 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Length of Time in Chair (min)	50	02/16/2024 8:50 AM PST	Your Scripps Health Team

Comfort and Environment Interventions

Question	Answer	Date of Assessment	Author
Comfort	Repositioned	02/17/2024 3:23 PM PST	Your Scripps Health Team
Restraint Alternatives	Bed alarms	02/17/2024 3:23 PM PST	Your Scripps Health Team
Special Mattress	Alternating pressure redistribution	02/17/2024 3:23 PM PST	Your Scripps Health Team
Additional Comfort/Environmental Interventions	Special mattress	02/17/2024 3:23 PM PST	Your Scripps Health Team

Entertainment

Question	Answer	Date of Assessment	Author
Entertainment Activities	Television	02/17/2024 3:23 PM PST	Your Scripps Health Team

Bedside Tablet Status

Question	Answer	Date of Assessment	Author
Bedside Tablet Active	Yes, hospital tablet	02/16/2024 7:35 PM PST	Your Scripps Health Team

Oncology

Question	Answer	Date of Assessment	Author
Does patient have an active cancer diagnosis?	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Discharge Planning (CM/SW)

Question	Answer	Date of Assessment	Author
Living Arrangements	Other	02/17/2024 11:58 AM PST	Your Scripps Health Team
Has discharge transport been arranged?	CM	02/17/2024 11:58 AM PST	Your Scripps Health Team
Transport company name and number	AMR WC shuttle	02/17/2024 11:58 AM PST	Your Scripps Health Team
What day is the transport expected?	66887	02/17/2024 11:58 AM PST	Your Scripps Health Team
What time is the transport expected?	50400	02/17/2024 11:58 AM PST	Your Scripps Health Team
Does the patient have someone to pick them up at discharge?	No	02/17/2024 11:58 AM PST	Your Scripps Health Team

Discharge Planning

Question	Answer	Date of Assessment	Author
Designated Family Caregiver	Documented in Emergency Contacts	01/19/2024 6:43 AM PST	Your Scripps Health Team
Home Care Services	No	01/24/2024 7:00 AM PST	Your Scripps Health Team
(Retired) Patient expects to be discharged to:	TBD	01/19/2024 6:43 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Support Systems	Family members	01/24/2024 7:00 AM PST	Your Scripps Health Team
(Retired) Assistance Needed	with housing situation, pt states unsafe at this time	01/19/2024 6:43 AM PST	Your Scripps Health Team
Where was pt living / residing	Homeless, unsheltered	01/24/2024 7:00 AM PST	Your Scripps Health Team
Prior assistance needed	None/ Denies Need	01/24/2024 7:00 AM PST	Your Scripps Health Team
Patient expects to be discharged to	Not Known	01/24/2024 7:00 AM PST	Your Scripps Health Team

Initial Discharge Planning

Question	Answer	Date of Assessment	Author
(Retired) Assistive Devices	None	01/13/2024 12:00 PM PST	Your Scripps Health Team

Confusion Assessment Method (ICU-CAM)

Question	Answer	Date of Assessment	Author
Feature 1: Acute Onset or Fluctuating Course	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team
Feature 2: Inattention	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team
Feature 4: Disorganized Thinking	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team
Overall CAM	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team

RUE Neurovascular Assessment

Question	Answer	Date of Assessment	Author
R Radial Pulse	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team

LUE Neurovascular Assessment

Question	Answer	Date of Assessment	Author
L Radial Pulse	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team

RLE Neurovascular Assessment

Question	Answer	Date of Assessment	Author
R Posterior Tibial Pulse	+1	02/17/2024 7:15 AM PST	Your Scripps Health Team
R Dorsalis Pedis Pulse	Doppler	02/17/2024 7:15 AM PST	Your Scripps Health Team

LLE Neurovascular Assessment

Question	Answer	Date of Assessment	Author
L Posterior Tibial Pulse	+1	02/17/2024 7:15 AM PST	Your Scripps Health Team
L Dorsalis Pedis Pulse	Doppler	02/17/2024 7:15 AM PST	Your Scripps Health Team

Genitalia

Question	Answer	Date of Assessment	Author
Female Genitalia	Intact	02/16/2024 7:35 PM PST	Your Scripps Health Team

Genitourinary

Question	Answer	Date of Assessment	Author
Genitourinary Symptoms	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
Genitourinary (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team

Genitourinary Additional Assessments

Question	Answer	Date of Assessment	Author
Genitourinary Additional Assessments	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Cough and Deep Breathe

Question	Answer	Date of Assessment	Author
Cough and Deep Breathe	Yes	02/16/2024 7:35 PM PST	Your Scripps Health Team

Respiratory Interventions

Answer	Date of Assessment	Author
Cough and deep breathe	02/16/2024 7:35 PM PST	Your Scripps Health Team

ARS (Complete on admission, prior to resuming PO intake, and post procedure)

Question	Answer	Date of Assessment	Author
Is the Patient Intubated? (If yes, defer ARS for 4 hours after extubation)	0	01/14/2024 8:00 PM PST	Your Scripps Health Team

ARS-Contraindications for Water Test

Question	Answer	Date of Assessment	Author
Is the patient Strict NPO? (If yes, defer ARS until diet order placed)	0	01/14/2024 4:36 PM PST	Your Scripps Health Team
Admitted for Bariatric Surgery?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Are there current speech therapy orders for swallow evaluation of dysphagia?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Has the patient/caregiver reported a history of difficulty swallowing?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Does the patient have facial asymmetry?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team

ARS-Tier 1

Question	Answer	Date of Assessment	Author
Is the alertness level INsufficient to remain awake for 10 minutes while sitting upright?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Is voice weak, wet or abnormal in any way? (Yes if patient cannot speak)	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Does the patient drool?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Is speech slurred?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Is the patients cough weak or inaudible?, if no cough then YES	0	01/14/2024 8:00 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Aspiration Risk Result	Proceed to contraindications for water test	01/14/2024 8:00 PM PST	Your Scripps Health Team

ARS-Water Test Required - only if all the questions are 'no'

Question	Answer	Date of Assessment	Author
Water Test Required?	Water test required	01/14/2024 4:36 PM PST	Your Scripps Health Team

ARS-3 Ounce Water Test (Complete ONCE prior to PO intake)

Question	Answer	Date of Assessment	Author
Have the patient drink 3 oz of water without stopping. Does the patient choke, cough, throat clear or have a wet vocal quality up to one minute after?	Pass	01/14/2024 8:00 PM PST	Your Scripps Health Team

Intake

Question	Answer	Date of Assessment	Author
P.O.	240	02/17/2024 1:00 PM PST	Your Scripps Health Team
Snacks	ice cream, brownie	02/10/2024 11:55 AM PST	Your Scripps Health Team
Percent Meals Eaten (%)	75	02/17/2024 1:00 PM PST	Your Scripps Health Team
Meal	Lunch	02/17/2024 1:00 PM PST	Your Scripps Health Team
I.V.	10	02/17/2024 4:00 PM PST	Your Scripps Health Team
Meal - Number of Carbs Consumed (gm)	30	02/06/2024 4:40 PM PST	Your Scripps Health Team

Nutrition

Question	Answer	Date of Assessment	Author
Feeding	Able to feed self	02/17/2024 2:00 PM PST	Your Scripps Health Team

Social History

Question	Answer	Date of Assessment	Author
Type of Residence	Shelter	02/12/2024 10:59 AM PST	Kalpana S

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	X	02/16/2024 7:35 PM PST	Your Scripps Health Team
Traction in Use	No	02/16/2024 7:35 PM PST	Your Scripps Health Team
RLE	Swelling	02/16/2024 7:35 PM PST	Your Scripps Health Team
LLE	Swelling	02/16/2024 7:35 PM PST	Your Scripps Health Team
RUE	Full movement	02/16/2024 7:40 AM PST	Your Scripps Health Team
LUE	Full movement	02/16/2024 7:40 AM PST	Your Scripps Health Team
Musculoskeletal Additional Assessments	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Screen for MRSA (all items must be valued)

Question	Answer	Date of Assessment	Author
Admission to ICU	Yes	01/19/2024 6:39 AM PST	Your Scripps Health Team
Hospitalized within last 30 days	No	01/19/2024 6:39 AM PST	Your Scripps Health Team
Inpatient dialysis treatment	No	01/19/2024 6:39 AM PST	Your Scripps Health Team
Admit/Transfer from a Skilled Nursing Facility (SNF)	No	01/19/2024 6:39 AM PST	Your Scripps Health Team
Known history of MRSA	No	01/19/2024 6:39 AM PST	Your Scripps Health Team
Results and Interventions	Obtain Nares Sample - Order MRSA Surveillance (LAB8345)	01/19/2024 6:39 AM PST	Your Scripps Health Team

Trauma/Abuse Screening

Question	Answer	Date of Assessment	Author
Physical Abuse	Yes, present (Comment)	01/19/2024 6:39 AM PST	Your Scripps Health Team
Verbal Abuse	Yes, present (Comment)	01/19/2024 6:39 AM PST	Your Scripps Health Team
Sexual Abuse	Denies	01/19/2024 6:39 AM PST	Your Scripps Health Team

Breastfeeding Status

Answer	Date of Assessment	Author
No	01/19/2024 6:41 AM PST	Your Scripps Health Team

Social Drivers of Health Screening

Question	Answer	Date of Assessment	Author
What is your living situation today?	Do not have a stable place to live	01/19/2024 6:42 AM PST	Your Scripps Health Team
In the past 3 months, have utility companies threatened to shut off services in your home that would affect your health (healthcare equipment, water supply, refrigerator, heat, etc.)? (If you are unsheltered or homeless, please select "Not Applicable.")	Not Applicable	01/19/2024 6:42 AM PST	Your Scripps Health Team
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024 6:42 AM PST	Your Scripps Health Team
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024 6:42 AM PST	Your Scripps Health Team
In the past 3 months, has anyone, including your family, caregiver, or a friend, physically hurt you or threatened you?	Yes	01/19/2024 6:42 AM PST	Your Scripps Health Team

Assistance Levels - How much assistance does the patient need?

Question	Answer	Date of Assessment	Author
To advance to sitting edge of bed and to maintain sitting balance?	Requires greater than minimal assistance Stop the screen	01/19/2024 6:43 AM PST	Your Scripps Health Team
To safely advance to standing (with Gait belt and Assistive device as indicated)?	Requires greater than minimal assistance Stop the screen	01/19/2024 6:43 AM PST	Your Scripps Health Team
To march in place or to transfer to bedside chair/commode (with gait belt and Assistive Device as Indicated)?	Requires greater than minimal assistance Stop the screen	01/19/2024 6:43 AM PST	Your Scripps Health Team
To safely ambulate to bathroom or doorway (with gait belt and Assistive Device as indicated)?	Requires greater than minimal assistance Stop the screen	01/19/2024 6:43 AM PST	Your Scripps Health Team
Does the patient report that they are mobilizing at their functional baseline, without concerns for safe home mobility? (Retired)	No	01/19/2024 6:43 AM PST	Your Scripps Health Team

ADL Functional Risk Screen

Question	Answer	Date of Assessment	Author
OT - Recent Onset	2;3;4	01/19/2024 6:43 AM PST	Your Scripps Health Team
ST - Recent Onset	0	01/19/2024 6:43 AM PST	Your Scripps Health Team
Results (OT and SLP)	Notify MD for Evaluation Order	01/19/2024 6:43 AM PST	Your Scripps Health Team

Nurse Mobility Screen

Question	Answer	Date of Assessment	Author
Exclusion Critera	No Exclusion Criteria	01/19/2024 6:43 AM PST	Your Scripps Health Team

Mobility Screening Outcome

Question	Answer	Date of Assessment	Author
Mobility Screening Outcome(Retired)	Patient requires more than minimal assistance of 1 person. PT/OT evaluation indicated.	01/19/2024 6:43 AM PST	Your Scripps Health Team

Advance Directives (For Healthcare)

Question	Answer	Date of Assessment	Author
Have you reviewed your Advance Directive and is it current?	Patient does not have Advance Directive	01/19/2024 6:43 AM PST	Your Scripps Health Team
Advance Directive	Patient does not have advance directive;Patient would like information	01/19/2024 6:43 AM PST	Your Scripps Health Team
Information Provided on Healthcare Directives	Yes	01/19/2024 6:43 AM PST	Your Scripps Health Team
Healthcare Agent Appointed	No	01/19/2024 6:43 AM PST	Your Scripps Health Team
Pre-existing DNR/DNI Order	No	01/19/2024 6:43 AM PST	Your Scripps Health Team
Patient Requests Assistance	Inpatient Consult to Spiritual care placed	01/19/2024 6:43 AM PST	Your Scripps Health Team

Sleep Apnea Screening

Question	Answer	Date of Assessment	Author
Do you SNORE loudly?	0	01/19/2024 6:46 AM PST	Your Scripps Health Team
Do you often feel tired, fatigued, or sleepy during the daytime?	1	01/19/2024 6:46 AM PST	Your Scripps Health Team
Do you have or are you being treated for high blood pressure?	1	01/19/2024 6:46 AM PST	Your Scripps Health Team
Is patient obese/ very overweight – BMI more than 35 kg/m2?	0	01/19/2024 6:46 AM PST	Your Scripps Health Team
Age over 50 years old?	1	01/19/2024 6:46 AM PST	Your Scripps Health Team
Neck Circumference >16 inches?	0	01/19/2024 6:46 AM PST	Your Scripps Health Team
Is patient male?	0	01/19/2024 6:46 AM PST	Your Scripps Health Team
Has anyone observed you stop breathing during your sleep?	0	01/19/2024 6:46 AM PST	Your Scripps Health Team
Score	3	01/19/2024 6:46 AM PST	Your Scripps Health Team

Meds to Beds for Discharge Prescriptions

Question	Answer	Date of Assessment	Author
Will the patient be utilizing the meds to beds program?	No	01/19/2024 9:11 AM PST	Jennifer K

Malnutrition Screening Tool

Question	Answer	Date of Assessment	Author
Have you recently lost weight without trying?	2	01/19/2024 6:47 AM PST	Your Scripps Health Team
Weight Loss Score	2	01/19/2024 6:47 AM PST	Your Scripps Health Team
Have you been eating poorly because of your decreased appetite?	1	01/19/2024 6:47 AM PST	Your Scripps Health Team
Appetite Score	1	01/19/2024 6:47 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Malnutrition Screening Outcome	3	01/19/2024 6:47 AM PST	Your Scripps Health Team

Values/Beliefs

Question	Answer	Date of Assessment	Author
Cultural Practices During Hospitalization	none	01/19/2024 6:47 AM PST	Your Scripps Health Team
Spiritual Practices During Hospitalization	catholic	01/19/2024 6:47 AM PST	Your Scripps Health Team

OT Discharge Recommendations

Question	Answer	Date of Assessment	Author
OT Discharge Dispo Recommendations	Consult with CM/SW for Placement	02/12/2024 10:59 AM PST	Kalpana S
OT Safety Concerns	homelss; pain in R foot	02/02/2024 3:07 PM PST	Elaheh S
OT Discharge Assistive Device Needs	FWW - front wheeled walker	02/02/2024 3:07 PM PST	Elaheh S
Discharge Summary	No skilled OT intervention is recommended as patient appears to be at baseline	02/12/2024 10:59 AM PST	Kalpana S

6 Clicks Daily Activity OT

Question	Answer	Date of Assessment	Author
Putting on and taking off regular lower body clothing?	4	02/12/2024 10:59 AM PST	Kalpana S
Bathing (including washing, rinsing, drying?)	3	02/12/2024 10:59 AM PST	Kalpana S
Toileting, which includes using toilet, bedpan, or urinal?	4	02/12/2024 10:59 AM PST	Kalpana S
Putting on and taking off regular upper body clothing?	4	02/12/2024 10:59 AM PST	Kalpana S
Taking care of personal grooming such as brushing teeth?	4	02/12/2024 10:59 AM PST	Kalpana S
Eating meals?	4	02/12/2024 10:59 AM PST	Kalpana S
OT 6 Clicks Score	23	02/12/2024 10:59 AM PST	Kalpana S

Prior Level of Function

Question	Answer	Date of Assessment	Author
Functional Mobility	Independent	02/12/2024 10:59 AM PST	Kalpana S
ADLs	Independent	02/12/2024 10:59 AM PST	Kalpana S

Occupational Function

Question	Answer	Date of Assessment	Author
ADL/IADL (Y)	ADL	02/12/2024 10:59 AM PST	Kalpana S
Self Feeding	Independent	02/12/2024 10:59 AM PST	Kalpana S

Question	Answer	Date of Assessment	Author
Hygiene/Grooming	Independent	02/12/2024 10:59 AM PST	Kalpana S
Dressing UE	Independent	02/12/2024 10:59 AM PST	Kalpana S
Dressing LE	Independent	02/12/2024 10:59 AM PST	Kalpana S
Toileting/Pericare	Independent	02/12/2024 10:59 AM PST	Kalpana S
Sit to Stand	Independent	02/12/2024 10:59 AM PST	Kalpana S
Transfer Type	Stand pivot	02/12/2024 10:59 AM PST	Kalpana S

Bed Mobility Training

Question	Answer	Date of Assessment	Author
Supine to Sit	Independent	02/09/2024 10:05 AM PST	Megan G
Sit to Supine	Independent	02/05/2024 11:20 AM PST	Your Scripps Health Team
Rolling R	Independent	02/09/2024 10:05 AM PST	Megan G
Rolling L	Independent	02/09/2024 10:05 AM PST	Megan G
Sidelying to SIt	Modified Independent (extra time/ device/ safety)	01/23/2024 9:25 AM PST	Deborah S
Sit to Sidelying	Modified Independent (extra time/ device/ safety)	01/23/2024 9:25 AM PST	Deborah S

Functional Assessment

Question	Answer	Date of Assessment	Author
Stand to Sit	Independent	02/12/2024 10:59 AM PST	Kalpana S
Commode/Toilet Transfer	Independent	02/12/2024 10:59 AM PST	Kalpana S
Scooting	Modified Independent (extra time/ device/ safety)	02/06/2024 12:04 PM PST	Lora S

Balance

Question	Answer	Date of Assessment	Author
Sitting Static	Normal	02/12/2024 10:59 AM PST	Kalpana S
Sitting Dynamic	Normal	02/12/2024 10:59 AM PST	Kalpana S
Standing Static	Good	02/12/2024 10:59 AM PST	Kalpana S
Standing Dynamic	Good -	02/12/2024 10:59 AM PST	Kalpana S

Gait Training

Question	Answer	Date of Assessment	Author
Gait Assistance	Modified Independent (extra time/ device/ safety)	02/09/2024 10:05 AM PST	Megan G
Gait Distance (ft)	200	02/09/2024 10:05 AM PST	Megan G
Gait Assistive Device	4WW	02/09/2024 10:05 AM PST	Megan G
Stairs Assistive Device	FWW	02/02/2024 12:38 PM PST	Tania R

6 Clicks Basic Mobility PT

Question	Answer	Date of Assessment	Author
Turning from your back to your side while in a flat bed without using bedrails?	4	02/09/2024 10:05 AM PST	Megan G
Moving from lying on your back to sitting on the side of a flat bed without using bedrails?	4	02/09/2024 10:05 AM PST	Megan G
Moving to and from a bed to a chair (including a wheelchair)?	4	02/09/2024 10:05 AM PST	Megan G
Standing up from a chair using your arms (e.g., wheelchair or bedside chair)?	4	02/09/2024 10:05 AM PST	Megan G
Walking in hospital room?	4	02/09/2024 10:05 AM PST	Megan G
Climbing 3-5 steps with a railing?	3	02/09/2024 10:05 AM PST	Megan G
PT 6 Clicks Score	23	02/09/2024 10:05 AM PST	Megan G

PT Discharge Recommendations

Question	Answer	Date of Assessment	Author
PT Discharge Dispo Recommendations	Consult with CM/SW for Placement	02/09/2024 10:05 AM PST	Megan G
PT Discharge Assistive Device Needs	4WW	02/09/2024 10:05 AM PST	Megan G
PT Safety Concerns	fall risk without AD	02/09/2024 10:05 AM PST	Megan G
Discharge Summary	pt is mod I with a 4ww. I recommend she always use it due to hx of ankle/foot issues, general pain and improved stability with it. no further PT recommended. d/c PT. pt to ambulate with nursing assist and a walker.	02/09/2024 10:05 AM PST	Megan G

Incentive Spirometry

Question	Answer	Date of Assessment	Author
Incentive Spirometry Achieved (mL)	500	02/09/2024 9:22 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Incentive Spirometry Goal (mL)	2500	02/06/2024 9:00 PM PST	Your Scripps Health Team

Transfers

Question	Answer	Date of Assessment	Author
Bed to Chair	Independent	02/09/2024 10:05 AM PST	Megan G
Chair to Bed	Independent	02/09/2024 10:05 AM PST	Megan G

Emesis Output/Assessment

Question	Answer	Date of Assessment	Author
Emesis	0	02/04/2024 8:00 AM PST	Your Scripps Health Team

Oxygen Therapy/Pulse Ox

Question	Answer	Date of Assessment	Author
O2 Flow Rate (L/min)	0	02/15/2024 11:15 AM PST	Maria G

P/F Calculation

Question	Answer	Date of Assessment	Author
FiO2 (%)	21	02/15/2024 11:15 AM PST	Maria G

O2 Delivery Method

Answer	Date of Assessment	Author
Nasal cannula	02/17/2024 7:21 AM PST	Your Scripps Health Team

Oxygen Therapy

Question	Answer	Date of Assessment	Author
Patient Activity	At rest	02/17/2024 3:30 PM PST	Your Scripps Health Team
Pulse Oximetry Type	Continuous	02/15/2024 9:20 PM PST	Your Scripps Health Team
Oxygen Therapy	None (Room air)	02/17/2024 3:30 PM PST	Your Scripps Health Team
Vent Mode	PS/CPAP	01/14/2024 10:00 AM PST	Your Scripps Health Team

Provider Notification

Question	Answer	Date of Assessment	Author
Critical Lab Test Name	Other (comment)	02/04/2024 8:00 AM PST	Your Scripps Health Team
Critical Exam Test Name	Other (comment)	02/04/2024 8:00 AM PST	Your Scripps Health Team
Reason for Communication	Other (Comment)	02/15/2024 1:46 AM PST	Your Scripps Health Team
Provider Name	Jerry Chen	02/15/2024 1:46 AM PST	Your Scripps Health Team
Provider Role	Resident;Hospitalist	02/15/2024 1:46 AM PST	Your Scripps Health Team
Method of Communication	Secure chat	02/15/2024 1:46 AM PST	Your Scripps Health Team
Response	Other (Comment)	02/15/2024 1:46 AM PST	Your Scripps Health Team

Are rigors present?

Answer	Date of Assessment	Author
No	01/12/2024 7:58 PM PST	Your Scripps Health Team

Is there a suspected infection?

Answer	Date of Assessment	Author
No	01/12/2024 7:58 PM PST	Your Scripps Health Team

Is the patient's mental status altered?

Answer	Date of Assessment	Author
No	01/12/2024 7:58 PM PST	Your Scripps Health Team

BiPAP/NIV

Question	Answer	Date of Assessment	Author
IPAP (cm H2O)	14	01/13/2024 3:50 AM PST	Domingo P
FiO2 - NIV (%)	60	01/13/2024 3:20 AM PST	Domingo P
EPAP (cm H2O)	7	01/13/2024 3:50 AM PST	Domingo P
Insp Time NIV (sec)	0.8	01/13/2024 3:20 AM PST	Domingo P

Readings

Question	Answer	Date of Assessment	Author
PIP (cm H2O)	11	01/14/2024 11:11 AM PST	Ryan O
Vt Exp (mL)	340	01/14/2024 11:11 AM PST	Ryan O
Minute Ventilation (L/min)	9.5	01/14/2024 11:11 AM PST	Ryan O
MAP (cm H2O)	7	01/14/2024 11:11 AM PST	Ryan O
I:E Ratio	1:3.0	01/14/2024 7:30 AM PST	Ryan O
Static Compliance (mL/cm H2O)	34	01/14/2024 11:11 AM PST	Ryan O
Dynamic Compliance (mL/cm H2O)	22	01/13/2024 6:09 PM PST	Ji Y

Cares/ Safeties

Question	Answer	Date of Assessment	Author
Safety Assessments	Bag valve mask present;Ventilator plugged into red outlet	01/14/2024 7:30 AM PST	Ryan O

Respiratory

Question	Answer	Date of Assessment	Author
Respiratory Pattern	Normal	02/17/2024 7:15 AM PST	Your Scripps Health Team
Chest Assessment	Chest expansion symmetrical	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cough	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
R Upper Breath Sounds	Clear	02/17/2024 7:15 AM PST	Your Scripps Health Team
L Upper Breath Sounds	Clear	02/17/2024 7:15 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Bilateral Breath Sounds	Clear;Diminished	02/17/2024 7:15 AM PST	Your Scripps Health Team
Dyspnea Occurrence	With exertion	02/16/2024 7:35 PM PST	Your Scripps Health Team
R Basilar	Diminished	02/17/2024 7:15 AM PST	Your Scripps Health Team
L Basilar	Diminished	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ventilator Patient	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Airway LDA	Endotracheal tube	01/14/2024 8:00 AM PST	Your Scripps Health Team
Respiratory Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Respiratory Effort	Unlabored	02/17/2024 7:15 AM PST	Your Scripps Health Team
Respiratory Depth/Rhythm	Regular	02/17/2024 7:15 AM PST	Your Scripps Health Team
Respiratory (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team

Tuberculosis Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	Denies symptoms	01/12/2024 8:22 PM PST	Nurse Jesse T
Risks Present	Denies risks	01/12/2024 8:22 PM PST	Nurse Jesse T
Results	TB Risk Screen Negative	01/12/2024 8:22 PM PST	Nurse Jesse T

Morse Fall Risk

Question	Answer	Date of Assessment	Author
History of Falling	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Secondary Diagnosis	15	01/12/2024 8:22 PM PST	Nurse Jesse T
Ambulatory Aids	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Intravenous Therapy/Heparin/Saline Lock	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Gait/Transferring	10	01/12/2024 8:22 PM PST	Nurse Jesse T
Mental Status	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Morse Fall Risk Score	25	01/12/2024 8:22 PM PST	Nurse Jesse T

Columbia Suicide Severity Rating Scale

Question	Answer	Date of Assessment	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	01/12/2024 8:23 PM PST	Nurse Jesse T
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	01/12/2024 8:23 PM PST	Nurse Jesse T
2. In the past month have you actually had any thoughts of killing yourself?	No	01/12/2024 8:23 PM PST	Nurse Jesse T

Violence Risk Screening (Step 1)

Question	Answer	Date of Assessment	Author
Automatic Triggers	NA	02/16/2024 7:40 AM PST	Your Scripps Health Team
Risk Factor Triggers	Substance Abuse	02/16/2024 7:40 AM PST	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Date of Assessment	Author
Violence Screening Outcome	Negative Violence Risk Screen	02/16/2024 7:40 AM PST	Your Scripps Health Team

Precaution Necessity Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Chlorhexidine Daily Bath Results	Initiate CHG bathing daily (not to be applied to open skin areas).	01/12/2024 9:29 PM PST	Nurse Jesse T
Precautions	Follow Standard Precautions	01/12/2024 9:29 PM PST	Nurse Jesse T
Patient Status	Homeless	01/12/2024 9:29 PM PST	Nurse Jesse T
Known Infection Status	None	01/12/2024 9:29 PM PST	Nurse Jesse T
Patient has an Open Draining Wound that cannot be contained by dressing	No	01/12/2024 9:29 PM PST	Nurse Jesse T

C.difficile Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	No Active Diarrhea	01/12/2024 9:29 PM PST	Nurse Jesse T
Risks Present	None of the Above	01/12/2024 9:29 PM PST	Nurse Jesse T
Results	No C. difficile test necessary	01/12/2024 9:29 PM PST	Nurse Jesse T

CRO and Candida auris Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
History of overnight hospitalization and/or invasive procedure outside of the United States within last 12 months	No	01/12/2024 9:29 PM PST	Nurse Jesse T
History of LTAC/SNF/Subacute residency within the last 12 months	No	01/12/2024 9:29 PM PST	Nurse Jesse T
(RETIRED) Transferring from any healthcare facility in Nevada state (i.e. ACH, SNF, LTAC)	No	01/12/2024 9:29 PM PST	Nurse Jesse T
Known History of CRO (Carbapenem-resistant organism)	No	01/12/2024 9:29 PM PST	Nurse Jesse T
CRO Results and Interventions	Specimen not required.	01/12/2024 9:29 PM PST	Nurse Jesse T
Known History of Candida auris	No	01/12/2024 9:29 PM PST	Nurse Jesse T
Candida auris Results and Interventions	Specimen not required.	01/12/2024 9:29 PM PST	Nurse Jesse T

Valuables Sent to Safe/Secure Area

Question	Answer	Date of Assessment	Author
Patient Valuables Sent to Safe/Secure Area	No Valuables	02/17/2024 3:23 PM PST	Your Scripps Health Team

Patient Medications

Question	Answer	Date of Assessment	Author
Medications brought by patient?	No	02/15/2024 9:30 PM PST	Your Scripps Health Team
Disposition of Medications	Sent to pharmacy	02/13/2024 8:00 PM PST	Nurse Prescilla S

HEENT

Question	Answer	Date of Assessment	Author
R Eye	Moderately impaired vision	02/16/2024 7:35 PM PST	Your Scripps Health Team
L Eye	Moderately impaired vision	02/16/2024 7:35 PM PST	Your Scripps Health Team
Throat	Other (Comment)	02/11/2024 8:30 PM PST	Your Scripps Health Team
Mucous Membrane(s)	Other (Comment)	02/11/2024 8:30 PM PST	Your Scripps Health Team
Teeth	Missing teeth	02/16/2024 7:35 PM PST	Your Scripps Health Team
HEENT (WDL)	X	02/16/2024 7:35 PM PST	Your Scripps Health Team
Lips	Dry	02/14/2024 8:05 PM PST	Your Scripps Health Team

HEENT

Question	Answer	Date of Assessment	Author
HEENT (WDL)	X	01/12/2024 8:23 PM PST	Nurse Jesse T

Endocrine

Question	Answer	Date of Assessment	Author
Endocrine (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team
Continuous Glucose Monitor in Use	No	02/15/2024 7:30 AM PST	Your Scripps Health Team
Needs Further Intervention for Glucose Control	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Other Active Endocrine Issue:	No	02/15/2024 7:30 AM PST	Your Scripps Health Team
(RETIRED, READ-ONLY) Rationale for Intervention Needed for Glucose Control	Other (Comment)	02/15/2024 7:30 AM PST	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	01/12/2024 8:23 PM PST	Nurse Jesse T

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	01/12/2024 8:23 PM PST	Nurse Jesse T

Vital Signs

Question	Answer	Date of Assessment	Author
EKG Rhythm	Sinus Rhythm	01/12/2024 9:34 PM PST	Nurse Jesse T

Pain Assessment

Question	Answer	Date of Assessment	Author
Pain Assessment	0-10	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Score	7	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Orientation	Right;Left	02/17/2024 2:21 AM PST	Your Scripps Health Team
Pain Interventions	Medication (See MAR);Repositioned;Rest	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Descriptors	Discomfort	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Onset	Ongoing	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Frequency	Intermittent	02/17/2024 3:48 PM PST	Your Scripps Health Team
Patient's Stated Pain Goal	No pain	02/17/2024 3:48 PM PST	Your Scripps Health Team
Response to Interventions	Effective per pt	02/17/2024 6:51 AM PST	Your Scripps Health Team
Pain Aggravating Factors	Reposition/movement	02/17/2024 3:48 PM PST	Your Scripps Health Team
Effect of Pain on Daily Activities	Activity	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Type	Chronic pain	02/17/2024 3:48 PM PST	Your Scripps Health Team
Clinical Progression of Pain	Gradually improving	02/17/2024 2:21 AM PST	Your Scripps Health Team
VDS Pain Score	No Pain (Pain Level 0)	02/17/2024 6:51 AM PST	Your Scripps Health Team
Pain Location	Back	02/17/2024 3:48 PM PST	Your Scripps Health Team
Wong-Baker FACES Pain Rating	0	02/16/2024 4:22 PM PST	Your Scripps Health Team

MAP (Monitor)

Answer	Date of Assessment	Author
90	02/17/2024 7:21 AM PST	Your Scripps Health Team

Charting Type

Question	Answer	Date of Assessment	Author
Charting Type	Shift assessment	02/17/2024 7:15 AM PST	Your Scripps Health Team

Hygiene

Question	Answer	Date of Assessment	Author
Chlorhexidine Bath	Yes	02/16/2024 9:37 PM PST	Your Scripps Health Team
Oral Care	Other (Comment)	02/15/2024 2:00 PM PST	Your Scripps Health Team
Level of Assistance	Independent	02/17/2024 3:23 PM PST	Your Scripps Health Team
Hygiene	Other (Comment)	02/15/2024 2:00 PM PST	Your Scripps Health Team
Skin Care	Moisture barrier	02/10/2024 8:00 PM PST	Your Scripps Health Team
Urethral Catheter Care	Yes	02/03/2024 4:00 AM PST	Your Scripps Health Team

Sedation Scales

Question	Answer	Date of Assessment	Author
Target Arousal Level (RASS)	-2	01/13/2024 2:36 PM PST	Pedro Dammert Coronado, MD
Pasero Opioid-Induced Sedation Scale (POSS)	1	02/17/2024 3:48 PM PST	Your Scripps Health Team
Richmond Agitation Sedation Scale (RASS)	0	02/17/2024 3:48 PM PST	Your Scripps Health Team

P/F Ratio

Answer	Date of Assessment	Author
0	02/17/2024 3:07 PM PST	Background I

Feature 3: Altered Level of Consciousness

Answer	Date of Assessment	Author
Negative	02/17/2024 3:48 PM PST	Your Scripps Health Team

Pain Radiating Towards

Answer	Date of Assessment	Author
Neck	02/16/2024 3:52 PM PST	Your Scripps Health Team

Pain Alleviating Factors

Answer	Date of Assessment	Author
Meds	02/16/2024 3:52 PM PST	Your Scripps Health Team

Glasgow Coma Scale

Question	Answer	Date of Assessment	Author
Eye Opening	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Best Verbal Response	5	02/17/2024 7:15 AM PST	Your Scripps Health Team
Best Motor Response	6	02/17/2024 7:15 AM PST	Your Scripps Health Team
Glasgow Coma Scale Score	15	02/17/2024 7:15 AM PST	Your Scripps Health Team

Neurological

Question	Answer	Date of Assessment	Author
Level of Consciousness	Alert	02/17/2024 7:15 AM PST	Your Scripps Health Team
Orientation Level	Oriented X4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cognition/Basic Behavior	Appropriate judgement;Follows commands	02/17/2024 7:15 AM PST	Your Scripps Health Team
Speech	Clear	02/17/2024 7:15 AM PST	Your Scripps Health Team
Neuro Symptoms	Anxiety	02/17/2024 7:15 AM PST	Your Scripps Health Team
Neuro Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Pupil Assessment	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Motor Function/Sensation Assessment	Grip;Dorsiflexion;Plantar flexion	02/16/2024 7:40 AM PST	Your Scripps Health Team
Neuro (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
L Pupil Reaction	Brisk	02/07/2024 8:15 AM PST	Nurse Feliccia T
L Pupil Size (mm)	2	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Pupil Reaction	Brisk	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Pupil Size (mm)	2	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Pupil Shape	Round	02/07/2024 8:15 AM PST	Nurse Feliccia T
L Pupil Shape	Round	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Hand Grip	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
L Hand Grip	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
Relieved by	Rest	02/17/2024 7:15 AM PST	Your Scripps Health Team
R Foot Dorsiflexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
L Foot Dorsiflexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
R Foot Plantar Flexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
L Foot Plantar Flexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
RUE Motor Strength	5-Normal muscle strength against full resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
LUE Motor Strength	5-Normal muscle strength against full resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
RLE Motor Strength	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
LLE Motor Strength	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
LUE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
LUE Sensation	Full sensation	02/16/2024 7:40 AM PST	Your Scripps Health Team
LLE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
LLE Sensation	Decreased	02/16/2024 7:40 AM PST	Your Scripps Health Team
RUE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
RUE Sensation	Full sensation	02/16/2024 7:40 AM PST	Your Scripps Health Team
RLE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
RLE Sensation	Decreased	02/16/2024 7:40 AM PST	Your Scripps Health Team

Pacemaker

Question	Answer	Date of Assessment	Author
Pacemaker	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Cardiac

Question	Answer	Date of Assessment	Author
Ectopy	Premature ventricular contractions	02/06/2024 9:00 PM PST	Your Scripps Health Team
Ectopy Frequency	Occasional	02/06/2024 9:00 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Heart Sounds	S1, S2	02/16/2024 7:40 AM PST	Your Scripps Health Team
Jugular Venous Distention (JVD)	No	02/12/2024 7:50 AM PST	Your Scripps Health Team
Cardiac Symptoms	None	02/12/2024 7:50 AM PST	Your Scripps Health Team
QTc (seconds)	433	02/06/2024 8:23 AM PST	Your Scripps Health Team
Cardiac (WDL)	X	02/12/2024 7:50 AM PST	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Date of Assessment	Author
Skin Color	Appropriate for ethnicity	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin Condition/Temp	Warm;Dry	02/17/2024 7:15 AM PST	Your Scripps Health Team

Skin

Question	Answer	Date of Assessment	Author
Skin Integrity	Redness;Scaling	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin Location	both feet	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin Turgor	Non-tenting	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team

Peripheral Vascular

Question	Answer	Date of Assessment	Author
Capillary Refill	Less than/equal to 2 seconds (All extremities)	02/17/2024 7:15 AM PST	Your Scripps Health Team
Pulses	R radial;L radial;R dorsalis pedis;L dorsalis pedis	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cyanosis	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
Edema	Generalized;Right lower extremity;Left lower extremity	02/17/2024 7:15 AM PST	Your Scripps Health Team
PVS Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Peripheral Vascular (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team
Generalized Edema	Non-pitting	02/17/2024 7:15 AM PST	Your Scripps Health Team
RLE Edema	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team
LLE Edema	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team

Gastrointestinal

Question	Answer	Date of Assessment	Author
Last BM Date	66886	02/17/2024 7:15 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Passing Flatus	Yes	02/17/2024 7:15 AM PST	Your Scripps Health Team
Palpation	Soft;No guarding;Nontender	02/17/2024 7:15 AM PST	Your Scripps Health Team
Bowel Sounds (All Quadrants)	Passing flatus	02/17/2024 7:15 AM PST	Your Scripps Health Team
Abdomen Inspection	Nondistended	02/17/2024 7:15 AM PST	Your Scripps Health Team
GI Symptoms	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
Gastrointestinal Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Gastrointestinal (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team

Stool Assessment

Question	Answer	Date of Assessment	Author
Bowel Incontinence	No	02/16/2024 7:40 AM PST	Your Scripps Health Team

Output (mL)

Question	Answer	Date of Assessment	Author
Stool Consistency (Bristol Stool Scale)	4-Like a sausage or snake, smooth and soft	02/10/2024 1:48 AM PST	Your Scripps Health Team
Stool Appearance	Unable to assess	02/10/2024 1:48 AM PST	Your Scripps Health Team
Stool Color	Green;Brown	02/10/2024 1:48 AM PST	Your Scripps Health Team
Urine Color	Yellow/straw	02/17/2024 11:00 AM PST	Your Scripps Health Team
Urine Appearance	Clear	02/17/2024 11:00 AM PST	Your Scripps Health Team
Urine Odor	Unable to assess	02/17/2024 11:00 AM PST	Your Scripps Health Team

Stool Output/Assessment

Question	Answer	Date of Assessment	Author
Stool Amount	Unable to assess	02/16/2024 4:00 PM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Date of Assessment	Author
Patient Behaviors/Mood	Calm;Cooperative	02/17/2024 7:15 AM PST	Your Scripps Health Team
Needs Expressed	Physical	02/17/2024 7:15 AM PST	Your Scripps Health Team
Psychosocial Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Express Feelings	Able to express	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Express Needs	Able to express	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Express Thoughts	Able to express	02/17/2024 7:15 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Ability to Understand Others	Understands	02/17/2024 7:15 AM PST	Your Scripps Health Team
Psychosocial (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team
Length of Time/Family Visitation	5-6 hrs	01/16/2024 8:00 AM PST	Your Scripps Health Team

Tattoos/Piercings

Question	Answer	Date of Assessment	Author
Does patient have tattoos?	No	02/15/2024 9:30 PM PST	Your Scripps Health Team
Piercings Remaining	No	02/15/2024 9:30 PM PST	Your Scripps Health Team

Braden Scale

Question	Answer	Date of Assessment	Author
Sensory Perceptions	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Moisture	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Activity	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Mobility	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Nutrition	3	02/17/2024 7:15 AM PST	Your Scripps Health Team
Friction and Shear	3	02/17/2024 7:15 AM PST	Your Scripps Health Team
Braden Scale Score	22	02/17/2024 7:15 AM PST	Your Scripps Health Team

Anus/Rectum

Question	Answer	Date of Assessment	Author
Anus/Rectum Observations	WDL	02/16/2024 7:35 PM PST	Your Scripps Health Team

Hester Davis Fall Risk Assessment

Question	Answer	Date of Assessment	Author
Mental Status/LOC/Awareness	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Communication/Sensory	1	02/17/2024 7:15 AM PST	Your Scripps Health Team
Behavior	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Mobility	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Toileting Needs	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Patient has IV Fluids or Tube Feeds	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Last Known Fall	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
NPO Greater than 24 hours?	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Nausea/Vomiting?	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Diuretics	3	02/17/2024 7:15 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Chemotherapy in the last month	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cardiac/CNS meds?	2	02/17/2024 7:15 AM PST	Your Scripps Health Team
Patient has low blood glucose or sodium	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Hester Davis Fall Risk Score	8	02/17/2024 7:15 AM PST	Your Scripps Health Team

Urine Output/Assessment

Question	Answer	Date of Assessment	Author
Bladder Scan Volume (mL)	214	01/14/2024 5:55 AM PST	Your Scripps Health Team
Urinary Incontinence	No	02/17/2024 4:00 PM PST	Your Scripps Health Team
Intermittent/Straight Cath (mL)	600	01/13/2024 11:35 PM PST	Your Scripps Health Team
Post Void Residual (mL)	0	01/13/2024 11:40 PM PST	Your Scripps Health Team

Precautions

Question	Answer	Date of Assessment	Author
Sacral Dressing	Applied	02/16/2024 6:00 AM PST	Your Scripps Health Team
Precautions	Fall risk	02/17/2024 3:23 PM PST	Your Scripps Health Team

Safe Environment

Question	Answer	Date of Assessment	Author
Safe Environment	Reviewed & Completed	02/17/2024 3:23 PM PST	Your Scripps Health Team

Assistive Devices at Bedside

Question	Answer	Date of Assessment	Author
Patient Assistive Devices at Bedside	Dentures;Vision	02/17/2024 3:23 PM PST	Your Scripps Health Team
Dentures	Uppers	02/17/2024 3:23 PM PST	Your Scripps Health Team
Vision - Corrective Lenses	Glasses	02/17/2024 3:23 PM PST	Your Scripps Health Team
Comment Assistive Devices at Beds	cell phone at bedside	02/16/2024 8:50 AM PST	Your Scripps Health Team

Family/Significant Other Communication

Question	Answer	Date of Assessment	Author
Did you engage the family?	No Family Identified	02/13/2024 8:00 PM PST	Nurse Prescilla S
How was family/ significant other updated?	Face to Face	01/16/2024 8:00 PM PST	Your Scripps Health Team
How did you engage the family?	Update on Patient Status	01/16/2024 8:00 PM PST	Your Scripps Health Team

Vent Settings and Readings

Question	Answer	Date of Assessment	Author
Resp Rate (Set)	20	01/14/2024 8:00 AM PST	Your Scripps Health Team
Vt (Set, mL)	400	01/14/2024 8:00 AM PST	Your Scripps Health Team

Settings

Question	Answer	Date of Assessment	Author
PEEP/CPAP (cm H2O)	5	01/14/2024 11:11 AM PST	Ryan O
Insp Time (sec)	0.74	01/14/2024 7:30 AM PST	Ryan O

Vital Signs

Question	Answer	Date of Assessment	Author
ETCO2 (Monitor)	44	01/26/2024 10:00 PM PST	Your Scripps Health Team

Cough/Sputum Description

Question	Answer	Date of Assessment	Author
Sputum Amount	None	02/15/2024 9:30 PM PST	Your Scripps Health Team
Sputum Color	None	02/11/2024 8:30 PM PST	Your Scripps Health Team
Sputum Consistency	None	02/07/2024 8:15 AM PST	Nurse Feliccia T
Sputum How Obtained	Spontaneous cough	01/23/2024 8:30 PM PST	Your Scripps Health Team
Cough Mechanism	Spontaneous	01/22/2024 7:40 AM PST	Your Scripps Health Team

Critical-Care Pain Observation Tool

Question	Answer	Date of Assessment	Author
Facial Expression	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Body Movements	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Compliance with the Ventilator (Intubated Patients)	0	01/14/2024 10:00 AM PST	Your Scripps Health Team
Vocalization (Extubated Patients)	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Muscle Tension	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Critical-Care Pain Observation Score	0	01/14/2024 12:00 PM PST	Your Scripps Health Team

Assistive Devices Sent Home

Question	Answer	Date of Assessment	Author
Patient Assistive Devices Sent Home	None	02/15/2024 9:30 PM PST	Your Scripps Health Team

Fall Risk Interventions

Question	Answer	Date of Assessment	Author
Fall signage in place	Yes	02/17/2024 3:23 PM PST	Your Scripps Health Team
Wristband fall alert on	Yes	02/17/2024 3:23 PM PST	Your Scripps Health Team
Room Door Open	Yes	02/17/2024 3:23 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Gait Belt Used	Not applicable	02/17/2024 3:23 PM PST	Your Scripps Health Team
Alarm On	Bed	02/17/2024 3:23 PM PST	Your Scripps Health Team
Other fall interventions	frerquent rounding	02/17/2024 3:23 PM PST	Your Scripps Health Team

Mobility

Question	Answer	Date of Assessment	Author
Mobility/Exercise Safety Screen	Proceed with mobilization - No exclusion criteria met	02/10/2024 9:23 AM PST	Your Scripps Health Team
Level of Assistance	Independent	02/17/2024 3:23 PM PST	Your Scripps Health Team
Mode of Transport	Ambulatory	02/17/2024 3:23 PM PST	Your Scripps Health Team
Repositioned	Turns self	02/17/2024 3:23 PM PST	Your Scripps Health Team
Range of Motion - Passive	All extremities	02/06/2024 8:23 AM PST	Your Scripps Health Team
Head of Bed Elevated	Self regulated	02/17/2024 12:00 AM PST	Your Scripps Health Team
Anti-Embolism Intervention (Intermittent Pneumatic Compression Stockings)	Refused	02/17/2024 7:15 AM PST	Your Scripps Health Team
Heels/Feet	Heels elevated off bed	02/17/2024 2:00 PM PST	Your Scripps Health Team
Anti-Embolism Devices	Sequential compression devices, below knee	02/17/2024 7:15 AM PST	Your Scripps Health Team
Anti-Embolism Device Location	Bilateral lower extremities	02/17/2024 7:15 AM PST	Your Scripps Health Team
Range of Motion - Active	All extremities	02/17/2024 12:00 AM PST	Your Scripps Health Team
$ Positioning Device Supply Charges	Boot Truvue (Heel Protectors)	02/07/2024 8:00 AM PST	Nurse Feliccia T
Assistive Device	None	02/14/2024 10:50 PM PST	Your Scripps Health Team
Mobility Level (Calculated)	Level 4	02/17/2024 3:23 PM PST	Your Scripps Health Team
Mobilization Response	Tolerated well	02/17/2024 12:00 AM PST	Your Scripps Health Team
Distance Ambulated (ft)	12	02/16/2024 7:35 PM PST	Your Scripps Health Team
Length of Time in Chair (min)	50	02/16/2024 8:50 AM PST	Your Scripps Health Team

Comfort and Environment Interventions

Question	Answer	Date of Assessment	Author
Comfort	Repositioned	02/17/2024 3:23 PM PST	Your Scripps Health Team
Restraint Alternatives	Bed alarms	02/17/2024 3:23 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Special Mattress	Alternating pressure redistribution	02/17/2024 3:23 PM PST	Your Scripps Health Team
Additional Comfort/Environmental Interventions	Special mattress	02/17/2024 3:23 PM PST	Your Scripps Health Team

Entertainment

Question	Answer	Date of Assessment	Author
Entertainment Activities	Television	02/17/2024 3:23 PM PST	Your Scripps Health Team

Bedside Tablet Status

Question	Answer	Date of Assessment	Author
Bedside Tablet Active	Yes, hospital tablet	02/16/2024 7:35 PM PST	Your Scripps Health Team

Interpreter Services

Question	Answer	Date of Assessment	Author
Communication Needs (Retired, Read-Only)	None	02/17/2024 11:58 AM PST	Your Scripps Health Team
Type of Resource Used (Retired, Read-Only)	Other (comment)	02/12/2024 10:59 AM PST	Kalpana S
Information Interpreted (Qualified Interpreter only)	Other (Comment);Plan of care	02/07/2024 3:41 PM PST	Your Scripps Health Team
Learner	Patient	02/12/2024 10:59 AM PST	Kalpana S

Oncology

Question	Answer	Date of Assessment	Author
Does patient have an active cancer diagnosis?	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Discharge Planning (CM/SW)

Question	Answer	Date of Assessment	Author
Living Arrangements	Other	02/17/2024 11:58 AM PST	Your Scripps Health Team
Has discharge transport been arranged?	CM	02/17/2024 11:58 AM PST	Your Scripps Health Team
Transport company name and number	AMR WC shuttle	02/17/2024 11:58 AM PST	Your Scripps Health Team
What day is the transport expected?	66887	02/17/2024 11:58 AM PST	Your Scripps Health Team
What time is the transport expected?	50400	02/17/2024 11:58 AM PST	Your Scripps Health Team
Does the patient have someone to pick them up at discharge?	No	02/17/2024 11:58 AM PST	Your Scripps Health Team

Discharge Planning

Question	Answer	Date of Assessment	Author
Designated Family Caregiver	Documented in Emergency Contacts	01/19/2024 6:43 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Home Care Services	No	01/24/2024 7:00 AM PST	Your Scripps Health Team
(Retired) Patient expects to be discharged to:	TBD	01/19/2024 6:43 AM PST	Your Scripps Health Team
Support Systems	Family members	01/24/2024 7:00 AM PST	Your Scripps Health Team
(Retired) Assistance Needed	with housing situation, pt states unsafe at this time	01/19/2024 6:43 AM PST	Your Scripps Health Team
Where was pt living / residing	Homeless, unsheltered	01/24/2024 7:00 AM PST	Your Scripps Health Team
Prior assistance needed	None/ Denies Need	01/24/2024 7:00 AM PST	Your Scripps Health Team
Patient expects to be discharged to	Not Known	01/24/2024 7:00 AM PST	Your Scripps Health Team

Initial Discharge Planning

Question	Answer	Date of Assessment	Author
(Retired) Assistive Devices	None	01/13/2024 12:00 PM PST	Your Scripps Health Team

Confusion Assessment Method (ICU-CAM)

Question	Answer	Date of Assessment	Author
Feature 1: Acute Onset or Fluctuating Course	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team
Feature 2: Inattention	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team
Feature 4: Disorganized Thinking	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team
Overall CAM	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team

RUE Neurovascular Assessment

Question	Answer	Date of Assessment	Author
R Radial Pulse	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team

LUE Neurovascular Assessment

Question	Answer	Date of Assessment	Author
L Radial Pulse	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team

RLE Neurovascular Assessment

Question	Answer	Date of Assessment	Author
R Posterior Tibial Pulse	+1	02/17/2024 7:15 AM PST	Your Scripps Health Team
R Dorsalis Pedis Pulse	Doppler	02/17/2024 7:15 AM PST	Your Scripps Health Team

LLE Neurovascular Assessment

Question	Answer	Date of Assessment	Author
L Posterior Tibial Pulse	+1	02/17/2024 7:15 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
L Dorsalis Pedis Pulse	Doppler	02/17/2024 7:15 AM PST	Your Scripps Health Team

Genitalia

Question	Answer	Date of Assessment	Author
Female Genitalia	Intact	02/16/2024 7:35 PM PST	Your Scripps Health Team

Genitourinary

Question	Answer	Date of Assessment	Author
Genitourinary Symptoms	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
Genitourinary (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team

Genitourinary Additional Assessments

Question	Answer	Date of Assessment	Author
Genitourinary Additional Assessments	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Cough and Deep Breathe

Question	Answer	Date of Assessment	Author
Cough and Deep Breathe	Yes	02/16/2024 7:35 PM PST	Your Scripps Health Team

Respiratory Interventions

Answer	Date of Assessment	Author
Cough and deep breathe	02/16/2024 7:35 PM PST	Your Scripps Health Team

ARS (Complete on admission, prior to resuming PO intake, and post procedure)

Question	Answer	Date of Assessment	Author
Is the Patient Intubated? (If yes, defer ARS for 4 hours after extubation)	0	01/14/2024 8:00 PM PST	Your Scripps Health Team

ARS-Contraindications for Water Test

Question	Answer	Date of Assessment	Author
Is the patient Strict NPO? (If yes, defer ARS until diet order placed)	0	01/14/2024 4:36 PM PST	Your Scripps Health Team
Admitted for Bariatric Surgery?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Are there current speech therapy orders for swallow evaluation of dysphagia?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Has the patient/caregiver reported a history of difficulty swallowing?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Does the patient have facial asymmetry?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team

ARS-Tier 1

Question	Answer	Date of Assessment	Author
Is the alertness level INsufficient to remain awake for 10 minutes while sitting upright?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Is voice weak, wet or abnormal in any way? (Yes if patient cannot speak)	0	01/14/2024 8:00 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Does the patient drool?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Is speech slurred?	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Is the patients cough weak or inaudible?, if no cough then YES	0	01/14/2024 8:00 PM PST	Your Scripps Health Team
Aspiration Risk Result	Proceed to contraindications for water test	01/14/2024 8:00 PM PST	Your Scripps Health Team

ARS-Water Test Required - only if all the questions are 'no'

Question	Answer	Date of Assessment	Author
Water Test Required?	Water test required	01/14/2024 4:36 PM PST	Your Scripps Health Team

Nutrition

Question	Answer	Date of Assessment	Author
Feeding	Able to feed self	02/17/2024 2:00 PM PST	Your Scripps Health Team

Social History

Question	Answer	Date of Assessment	Author
Type of Residence	Shelter	02/12/2024 10:59 AM PST	Kalpana S

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	X	02/16/2024 7:35 PM PST	Your Scripps Health Team
Traction in Use	No	02/16/2024 7:35 PM PST	Your Scripps Health Team
RLE	Swelling	02/16/2024 7:35 PM PST	Your Scripps Health Team
LLE	Swelling	02/16/2024 7:35 PM PST	Your Scripps Health Team
RUE	Full movement	02/16/2024 7:40 AM PST	Your Scripps Health Team
LUE	Full movement	02/16/2024 7:40 AM PST	Your Scripps Health Team
Musculoskeletal Additional Assessments	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Screen for MRSA (all items must be valued)

Question	Answer	Date of Assessment	Author
Admission to ICU	Yes	01/19/2024 6:39 AM PST	Your Scripps Health Team
Hospitalized within last 30 days	No	01/19/2024 6:39 AM PST	Your Scripps Health Team
Inpatient dialysis treatment	No	01/19/2024 6:39 AM PST	Your Scripps Health Team
Admit/Transfer from a Skilled Nursing Facility (SNF)	No	01/19/2024 6:39 AM PST	Your Scripps Health Team
Known history of MRSA	No	01/19/2024 6:39 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Results and Interventions	Obtain Nares Sample - Order MRSA Surveillance (LAB8345)	01/19/2024 6:39 AM PST	Your Scripps Health Team

Trauma/Abuse Screening

Question	Answer	Date of Assessment	Author
Physical Abuse	Yes, present (Comment)	01/19/2024 6:39 AM PST	Your Scripps Health Team
Verbal Abuse	Yes, present (Comment)	01/19/2024 6:39 AM PST	Your Scripps Health Team
Sexual Abuse	Denies	01/19/2024 6:39 AM PST	Your Scripps Health Team

Social Drivers of Health Screening

Question	Answer	Date of Assessment	Author
What is your living situation today?	Do not have a stable place to live	01/19/2024 6:42 AM PST	Your Scripps Health Team
In the past 3 months, have utility companies threatened to shut off services in your home that would affect your health (healthcare equipment, water supply, refrigerator, heat, etc.)? (If you are unsheltered or homeless, please select "Not Applicable.")	Not Applicable	01/19/2024 6:42 AM PST	Your Scripps Health Team
In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?	Yes	01/19/2024 6:42 AM PST	Your Scripps Health Team
In the past 3 months, has lack of transportation kept you from medical appointments or getting things you need that are essential to your health?	Yes	01/19/2024 6:42 AM PST	Your Scripps Health Team
In the past 3 months, has anyone, including your family, caregiver, or a friend, physically hurt you or threatened you?	Yes	01/19/2024 6:42 AM PST	Your Scripps Health Team

Assistance Levels - How much assistance does the patient need?

Question	Answer	Date of Assessment	Author
To advance to sitting edge of bed and to maintain sitting balance?	Requires greater than minimal assistance Stop the screen	01/19/2024 6:43 AM PST	Your Scripps Health Team
To safely advance to standing (with Gait belt and Assistive device as indicated)?	Requires greater than minimal assistance Stop the screen	01/19/2024 6:43 AM PST	Your Scripps Health Team
To march in place or to transfer to bedside chair/commode (with gait belt and Assistive Device as Indicated)?	Requires greater than minimal assistance Stop the screen	01/19/2024 6:43 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
To safely ambulate to bathroom or doorway (with gait belt and Assistive Device as indicated)?	Requires greater than minimal assistance Stop the screen	01/19/2024 6:43 AM PST	Your Scripps Health Team

ADL Functional Risk Screen

Question	Answer	Date of Assessment	Author
OT - Recent Onset	2;3;4	01/19/2024 6:43 AM PST	Your Scripps Health Team
ST - Recent Onset	0	01/19/2024 6:43 AM PST	Your Scripps Health Team
Results (OT and SLP)	Notify MD for Evaluation Order	01/19/2024 6:43 AM PST	Your Scripps Health Team

Nurse Mobility Screen

Question	Answer	Date of Assessment	Author
Exclusion Critera	No Exclusion Criteria	01/19/2024 6:43 AM PST	Your Scripps Health Team

Mobility Screening Outcome

Question	Answer	Date of Assessment	Author
Mobility Screening Outcome(Retired)	Patient requires more than minimal assistance of 1 person. PT/OT evaluation indicated.	01/19/2024 6:43 AM PST	Your Scripps Health Team

Advance Directives (For Healthcare)

Question	Answer	Date of Assessment	Author
Have you reviewed your Advance Directive and is it current?	Patient does not have Advance Directive	01/19/2024 6:43 AM PST	Your Scripps Health Team
Advance Directive	Patient does not have advance directive;Patient would like information	01/19/2024 6:43 AM PST	Your Scripps Health Team
Information Provided on Healthcare Directives	Yes	01/19/2024 6:43 AM PST	Your Scripps Health Team
Healthcare Agent Appointed	No	01/19/2024 6:43 AM PST	Your Scripps Health Team
Pre-existing DNR/DNI Order	No	01/19/2024 6:43 AM PST	Your Scripps Health Team
Patient Requests Assistance	Inpatient Consult to Spiritual care placed	01/19/2024 6:43 AM PST	Your Scripps Health Team

Meds to Beds for Discharge Prescriptions

Question	Answer	Date of Assessment	Author
Will the patient be utilizing the meds to beds program?	No	01/19/2024 9:11 AM PST	Jennifer K

Values/Beliefs

Question	Answer	Date of Assessment	Author
Cultural Practices During Hospitalization	none	01/19/2024 6:47 AM PST	Your Scripps Health Team
Spiritual Practices During Hospitalization	catholic	01/19/2024 6:47 AM PST	Your Scripps Health Team

Vision

Question	Answer	Date of Assessment	Author
Current Vision	Does not wear glasses	01/20/2024 9:25 AM PST	Elaheh S

Hearing

Question	Answer	Date of Assessment	Author
Hearing	Intact	02/12/2024 10:59 AM PST	Kalpana S

Intake

Question	Answer	Date of Assessment	Author
I.V.	10	02/17/2024 4:00 PM PST	Your Scripps Health Team

Incentive Spirometry

Question	Answer	Date of Assessment	Author
Incentive Spirometry Achieved (mL)	500	02/09/2024 9:22 AM PST	Your Scripps Health Team
Incentive Spirometry Goal (mL)	2500	02/06/2024 9:00 PM PST	Your Scripps Health Team

Mental Status - documented in this encounter

Oxygen Therapy/Pulse Ox

Question	Answer	Entry Date	Author
O2 Flow Rate (L/min)	0	02/15/2024 11:15 AM PST	Maria G

P/F Calculation

Question	Answer	Entry Date	Author
FiO2 (%)	21	02/15/2024 11:15 AM PST	Maria G

O2 Delivery Method

Answer	Entry Date	Author
Nasal cannula	02/17/2024 7:21 AM PST	Your Scripps Health Team

Oxygen Therapy

Question	Answer	Entry Date	Author
Patient Activity	At rest	02/17/2024 3:30 PM PST	Your Scripps Health Team
Pulse Oximetry Type	Continuous	02/15/2024 9:20 PM PST	Your Scripps Health Team
Oxygen Therapy	None (Room air)	02/17/2024 3:30 PM PST	Your Scripps Health Team
Vent Mode	PS/CPAP	01/14/2024 10:00 AM PST	Your Scripps Health Team

Provider Notification

Question	Answer	Entry Date	Author
Critical Lab Test Name	Other (comment)	02/04/2024 8:00 AM PST	Your Scripps Health Team
Critical Exam Test Name	Other (comment)	02/04/2024 8:00 AM PST	Your Scripps Health Team
Reason for Communication	Other (Comment)	02/15/2024 1:46 AM PST	Your Scripps Health Team
Provider Name	Jerry Chen	02/15/2024 1:46 AM PST	Your Scripps Health Team
Provider Role	Resident;Hospitalist	02/15/2024 1:46 AM PST	Your Scripps Health Team
Method of Communication	Secure chat	02/15/2024 1:46 AM PST	Your Scripps Health Team
Response	Other (Comment)	02/15/2024 1:46 AM PST	Your Scripps Health Team

Are rigors present?

Answer	Entry Date	Author
No	01/12/2024 7:58 PM PST	Your Scripps Health Team

Is there a suspected infection?

Answer	Entry Date	Author
No	01/12/2024 7:58 PM PST	Your Scripps Health Team

Is the patient's mental status altered?

Answer	Entry Date	Author
No	01/12/2024 7:58 PM PST	Your Scripps Health Team

BiPAP/NIV

Question	Answer	Entry Date	Author
IPAP (cm H2O)	14	01/13/2024 3:50 AM PST	Domingo P
FiO2 - NIV (%)	60	01/13/2024 3:20 AM PST	Domingo P
EPAP (cm H2O)	7	01/13/2024 3:50 AM PST	Domingo P
Insp Time NIV (sec)	0.8	01/13/2024 3:20 AM PST	Domingo P

Readings

Question	Answer	Entry Date	Author
PIP (cm H2O)	11	01/14/2024 11:11 AM PST	Ryan O
Vt Exp (mL)	340	01/14/2024 11:11 AM PST	Ryan O
Minute Ventilation (L/min)	9.5	01/14/2024 11:11 AM PST	Ryan O
MAP (cm H2O)	7	01/14/2024 11:11 AM PST	Ryan O
I:E Ratio	1:3.0	01/14/2024 7:30 AM PST	Ryan O
Static Compliance (mL/cm H2O)	34	01/14/2024 11:11 AM PST	Ryan O
Dynamic Compliance (mL/cm H2O)	22	01/13/2024 6:09 PM PST	Ji Y

Respiratory

Question	Answer	Entry Date	Author
Respiratory Pattern	Normal	02/17/2024 7:15 AM PST	Your Scripps Health Team
Chest Assessment	Chest expansion symmetrical	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cough	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
R Upper Breath Sounds	Clear	02/17/2024 7:15 AM PST	Your Scripps Health Team
L Upper Breath Sounds	Clear	02/17/2024 7:15 AM PST	Your Scripps Health Team
Bilateral Breath Sounds	Clear;Diminished	02/17/2024 7:15 AM PST	Your Scripps Health Team
Dyspnea Occurrence	With exertion	02/16/2024 7:35 PM PST	Your Scripps Health Team
R Basilar	Diminished	02/17/2024 7:15 AM PST	Your Scripps Health Team
L Basilar	Diminished	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ventilator Patient	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Airway LDA	Endotracheal tube	01/14/2024 8:00 AM PST	Your Scripps Health Team
Respiratory Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Respiratory Effort	Unlabored	02/17/2024 7:15 AM PST	Your Scripps Health Team
Respiratory Depth/Rhythm	Regular	02/17/2024 7:15 AM PST	Your Scripps Health Team
Respiratory (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team

Morse Fall Risk

Question	Answer	Entry Date	Author
History of Falling	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Secondary Diagnosis	15	01/12/2024 8:22 PM PST	Nurse Jesse T
Ambulatory Aids	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Intravenous Therapy/Heparin/Saline Lock	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Gait/Transferring	10	01/12/2024 8:22 PM PST	Nurse Jesse T
Mental Status	0	01/12/2024 8:22 PM PST	Nurse Jesse T
Morse Fall Risk Score	25	01/12/2024 8:22 PM PST	Nurse Jesse T

Columbia Suicide Severity Rating Scale

Question	Answer	Entry Date	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	01/12/2024 8:23 PM PST	Nurse Jesse T
1. In the past month have you wished you were dead or wished you could go to sleep and not wake up?	No	01/12/2024 8:23 PM PST	Nurse Jesse T
2. In the past month have you actually had any thoughts of killing yourself?	No	01/12/2024 8:23 PM PST	Nurse Jesse T

Violence Risk Screening (Step 1)

Question	Answer	Entry Date	Author
Automatic Triggers	NA	02/16/2024 7:40 AM PST	Your Scripps Health Team
Risk Factor Triggers	Substance Abuse	02/16/2024 7:40 AM PST	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Entry Date	Author
Violence Screening Outcome	Negative Violence Risk Screen	02/16/2024 7:40 AM PST	Your Scripps Health Team

Endocrine

Question	Answer	Entry Date	Author
Endocrine (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team
Continuous Glucose Monitor in Use	No	02/15/2024 7:30 AM PST	Your Scripps Health Team
Needs Further Intervention for Glucose Control	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Other Active Endocrine Issue:	No	02/15/2024 7:30 AM PST	Your Scripps Health Team
(RETIRED, READ-ONLY) Rationale for Intervention Needed for Glucose Control	Other (Comment)	02/15/2024 7:30 AM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Entry Date	Author
Psychosocial (WDL)	WDL	01/12/2024 8:23 PM PST	Nurse Jesse T

Vital Signs

Question	Answer	Entry Date	Author
EKG Rhythm	Sinus Rhythm	01/12/2024 9:34 PM PST	Nurse Jesse T

Pain Assessment

Question	Answer	Entry Date	Author
Pain Assessment	0-10	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Score	7	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Orientation	Right;Left	02/17/2024 2:21 AM PST	Your Scripps Health Team
Pain Interventions	Medication (See MAR);Repositioned;Rest	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Descriptors	Discomfort	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Onset	Ongoing	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Frequency	Intermittent	02/17/2024 3:48 PM PST	Your Scripps Health Team
Patient's Stated Pain Goal	No pain	02/17/2024 3:48 PM PST	Your Scripps Health Team
Response to Interventions	Effective per pt	02/17/2024 6:51 AM PST	Your Scripps Health Team
Pain Aggravating Factors	Reposition/movement	02/17/2024 3:48 PM PST	Your Scripps Health Team
Effect of Pain on Daily Activities	Activity	02/17/2024 3:48 PM PST	Your Scripps Health Team
Pain Type	Chronic pain	02/17/2024 3:48 PM PST	Your Scripps Health Team
Clinical Progression of Pain	Gradually improving	02/17/2024 2:21 AM PST	Your Scripps Health Team
VDS Pain Score	No Pain (Pain Level 0)	02/17/2024 6:51 AM PST	Your Scripps Health Team
Pain Location	Back	02/17/2024 3:48 PM PST	Your Scripps Health Team
Wong-Baker FACES Pain Rating	0	02/16/2024 4:22 PM PST	Your Scripps Health Team

MAP (Monitor)

Answer	Entry Date	Author
90	02/17/2024 7:21 AM PST	Your Scripps Health Team

Charting Type

Question	Answer	Entry Date	Author
Charting Type	Shift assessment	02/17/2024 7:15 AM PST	Your Scripps Health Team

Sedation Scales

Question	Answer	Entry Date	Author
Target Arousal Level (RASS)	-2	01/13/2024 2:36 PM PST	Pedro Dammert Coronado, MD
Pasero Opioid-Induced Sedation Scale (POSS)	1	02/17/2024 3:48 PM PST	Your Scripps Health Team
Richmond Agitation Sedation Scale (RASS)	0	02/17/2024 3:48 PM PST	Your Scripps Health Team

P/F Ratio

Answer	Entry Date	Author
0	02/17/2024 3:07 PM PST	Background I

Feature 3: Altered Level of Consciousness

Answer	Entry Date	Author
Negative	02/17/2024 3:48 PM PST	Your Scripps Health Team

Pain Radiating Towards

Answer	Entry Date	Author
Neck	02/16/2024 3:52 PM PST	Your Scripps Health Team

Pain Alleviating Factors

Answer	Entry Date	Author
Meds	02/16/2024 3:52 PM PST	Your Scripps Health Team

Glasgow Coma Scale

Question	Answer	Entry Date	Author
Eye Opening	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Best Verbal Response	5	02/17/2024 7:15 AM PST	Your Scripps Health Team
Best Motor Response	6	02/17/2024 7:15 AM PST	Your Scripps Health Team
Glasgow Coma Scale Score	15	02/17/2024 7:15 AM PST	Your Scripps Health Team

Neurological

Question	Answer	Entry Date	Author
Level of Consciousness	Alert	02/17/2024 7:15 AM PST	Your Scripps Health Team
Orientation Level	Oriented X4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cognition/Basic Behavior	Appropriate judgement;Follows commands	02/17/2024 7:15 AM PST	Your Scripps Health Team
Speech	Clear	02/17/2024 7:15 AM PST	Your Scripps Health Team
Neuro Symptoms	Anxiety	02/17/2024 7:15 AM PST	Your Scripps Health Team
Neuro Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Pupil Assessment	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Motor Function/Sensation Assessment	Grip;Dorsiflexion;Plantar flexion	02/16/2024 7:40 AM PST	Your Scripps Health Team
Neuro (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team
L Pupil Reaction	Brisk	02/07/2024 8:15 AM PST	Nurse Feliccia T
L Pupil Size (mm)	2	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Pupil Reaction	Brisk	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Pupil Size (mm)	2	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Pupil Shape	Round	02/07/2024 8:15 AM PST	Nurse Feliccia T
L Pupil Shape	Round	02/07/2024 8:15 AM PST	Nurse Feliccia T
R Hand Grip	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
L Hand Grip	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
Relieved by	Rest	02/17/2024 7:15 AM PST	Your Scripps Health Team
R Foot Dorsiflexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team

Question	Answer	Entry Date	Author
L Foot Dorsiflexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
R Foot Plantar Flexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
L Foot Plantar Flexion	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
RUE Motor Strength	5-Normal muscle strength against full resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
LUE Motor Strength	5-Normal muscle strength against full resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
RLE Motor Strength	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
LLE Motor Strength	4-Full range of motion against mild resistance	02/16/2024 7:40 AM PST	Your Scripps Health Team
LUE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
LLE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
RUE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team
RLE Motor Response	Responds to commands	02/16/2024 7:40 AM PST	Your Scripps Health Team

Pacemaker

Question	Answer	Entry Date	Author
Pacemaker	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Cardiac

Question	Answer	Entry Date	Author
Ectopy	Premature ventricular contractions	02/06/2024 9:00 PM PST	Your Scripps Health Team
Ectopy Frequency	Occasional	02/06/2024 9:00 PM PST	Your Scripps Health Team
Heart Sounds	S1, S2	02/16/2024 7:40 AM PST	Your Scripps Health Team
Jugular Venous Distention (JVD)	No	02/12/2024 7:50 AM PST	Your Scripps Health Team
Cardiac Symptoms	None	02/12/2024 7:50 AM PST	Your Scripps Health Team
QTc (seconds)	433	02/06/2024 8:23 AM PST	Your Scripps Health Team
Cardiac (WDL)	X	02/12/2024 7:50 AM PST	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Entry Date	Author
Skin Color	Appropriate for ethnicity	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin Condition/Temp	Warm;Dry	02/17/2024 7:15 AM PST	Your Scripps Health Team

Skin

Question	Answer	Entry Date	Author
Skin Integrity	Redness;Scaling	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin Location	both feet	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin Turgor	Non-tenting	02/17/2024 7:15 AM PST	Your Scripps Health Team
Skin (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team

Peripheral Vascular

Question	Answer	Entry Date	Author
Capillary Refill	Less than/equal to 2 seconds (All extremities)	02/17/2024 7:15 AM PST	Your Scripps Health Team
Pulses	R radial;L radial;R dorsalis pedis;L dorsalis pedis	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cyanosis	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
Edema	Generalized;Right lower extremity;Left lower extremity	02/17/2024 7:15 AM PST	Your Scripps Health Team
PVS Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Peripheral Vascular (WDL)	X	02/17/2024 7:15 AM PST	Your Scripps Health Team
Generalized Edema	Non-pitting	02/17/2024 7:15 AM PST	Your Scripps Health Team
RLE Edema	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team
LLE Edema	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team

Gastrointestinal

Question	Answer	Entry Date	Author
Last BM Date	66886	02/17/2024 7:15 AM PST	Your Scripps Health Team
Passing Flatus	Yes	02/17/2024 7:15 AM PST	Your Scripps Health Team
Palpation	Soft;No guarding;Nontender	02/17/2024 7:15 AM PST	Your Scripps Health Team
Bowel Sounds (All Quadrants)	Passing flatus	02/17/2024 7:15 AM PST	Your Scripps Health Team
Abdomen Inspection	Nondistended	02/17/2024 7:15 AM PST	Your Scripps Health Team
GI Symptoms	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
Gastrointestinal Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Gastrointestinal (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team

Stool Assessment

Question	Answer	Entry Date	Author
Bowel Incontinence	No	02/16/2024 7:40 AM PST	Your Scripps Health Team

Output (mL)

Question	Answer	Entry Date	Author
Stool Consistency (Bristol Stool Scale)	4-Like a sausage or snake, smooth and soft	02/10/2024 1:48 AM PST	Your Scripps Health Team
Stool Appearance	Unable to assess	02/10/2024 1:48 AM PST	Your Scripps Health Team
Stool Color	Green;Brown	02/10/2024 1:48 AM PST	Your Scripps Health Team
Urine Color	Yellow/straw	02/17/2024 11:00 AM PST	Your Scripps Health Team
Urine Appearance	Clear	02/17/2024 11:00 AM PST	Your Scripps Health Team
Urine Odor	Unable to assess	02/17/2024 11:00 AM PST	Your Scripps Health Team

Stool Output/Assessment

Question	Answer	Entry Date	Author
Stool Amount	Unable to assess	02/16/2024 4:00 PM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Entry Date	Author
Patient Behaviors/Mood	Calm;Cooperative	02/17/2024 7:15 AM PST	Your Scripps Health Team
Needs Expressed	Physical	02/17/2024 7:15 AM PST	Your Scripps Health Team
Psychosocial Additional Assessments	No	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Express Feelings	Able to express	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Express Needs	Able to express	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Express Thoughts	Able to express	02/17/2024 7:15 AM PST	Your Scripps Health Team
Ability to Understand Others	Understands	02/17/2024 7:15 AM PST	Your Scripps Health Team
Psychosocial (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team
Length of Time/Family Visitation	5-6 hrs	01/16/2024 8:00 AM PST	Your Scripps Health Team

HEENT

Question	Answer	Entry Date	Author
HEENT (WDL)	X	02/16/2024 7:35 PM PST	Your Scripps Health Team
Lips	Dry	02/14/2024 8:05 PM PST	Your Scripps Health Team

Tattoos/Piercings

Question	Answer	Entry Date	Author
Does patient have tattoos?	No	02/15/2024 9:30 PM PST	Your Scripps Health Team
Piercings Remaining	No	02/15/2024 9:30 PM PST	Your Scripps Health Team

Braden Scale

Question	Answer	Entry Date	Author
Sensory Perceptions	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Moisture	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Activity	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Mobility	4	02/17/2024 7:15 AM PST	Your Scripps Health Team
Nutrition	3	02/17/2024 7:15 AM PST	Your Scripps Health Team
Friction and Shear	3	02/17/2024 7:15 AM PST	Your Scripps Health Team
Braden Scale Score	22	02/17/2024 7:15 AM PST	Your Scripps Health Team

Anus/Rectum

Question	Answer	Entry Date	Author
Anus/Rectum Observations	WDL	02/16/2024 7:35 PM PST	Your Scripps Health Team

Hester Davis Fall Risk Assessment

Question	Answer	Entry Date	Author
Mental Status/LOC/Awareness	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Communication/Sensory	1	02/17/2024 7:15 AM PST	Your Scripps Health Team
Behavior	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Mobility	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Toileting Needs	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Patient has IV Fluids or Tube Feeds	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Last Known Fall	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
NPO Greater than 24 hours?	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Nausea/Vomiting?	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Diuretics	3	02/17/2024 7:15 AM PST	Your Scripps Health Team
Chemotherapy in the last month	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Cardiac/CNS meds?	2	02/17/2024 7:15 AM PST	Your Scripps Health Team
Patient has low blood glucose or sodium	0	02/17/2024 7:15 AM PST	Your Scripps Health Team
Hester Davis Fall Risk Score	8	02/17/2024 7:15 AM PST	Your Scripps Health Team

Urine Output/Assessment

Question	Answer	Entry Date	Author
Bladder Scan Volume (mL)	214	01/14/2024 5:55 AM PST	Your Scripps Health Team
Urinary Incontinence	No	02/17/2024 4:00 PM PST	Your Scripps Health Team

Question	Answer	Entry Date	Author
Intermittent/Straight Cath (mL)	600	01/13/2024 11:35 PM PST	Your Scripps Health Team
Post Void Residual (mL)	0	01/13/2024 11:40 PM PST	Your Scripps Health Team

Vent Settings and Readings

Question	Answer	Entry Date	Author
Resp Rate (Set)	20	01/14/2024 8:00 AM PST	Your Scripps Health Team
Vt (Set, mL)	400	01/14/2024 8:00 AM PST	Your Scripps Health Team

Settings

Question	Answer	Entry Date	Author
PEEP/CPAP (cm H2O)	5	01/14/2024 11:11 AM PST	Ryan O
Insp Time (sec)	0.74	01/14/2024 7:30 AM PST	Ryan O

Vital Signs

Question	Answer	Entry Date	Author
ETCO2 (Monitor)	44	01/26/2024 10:00 PM PST	Your Scripps Health Team

Cough/Sputum Description

Question	Answer	Entry Date	Author
Sputum Amount	None	02/15/2024 9:30 PM PST	Your Scripps Health Team
Sputum Color	None	02/11/2024 8:30 PM PST	Your Scripps Health Team
Sputum Consistency	None	02/07/2024 8:15 AM PST	Nurse Feliccia T
Sputum How Obtained	Spontaneous cough	01/23/2024 8:30 PM PST	Your Scripps Health Team
Cough Mechanism	Spontaneous	01/22/2024 7:40 AM PST	Your Scripps Health Team

Critical-Care Pain Observation Tool

Question	Answer	Entry Date	Author
Facial Expression	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Body Movements	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Compliance with the Ventilator (Intubated Patients)	0	01/14/2024 10:00 AM PST	Your Scripps Health Team
Vocalization (Extubated Patients)	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Muscle Tension	0	01/14/2024 12:00 PM PST	Your Scripps Health Team
Critical-Care Pain Observation Score	0	01/14/2024 12:00 PM PST	Your Scripps Health Team

Oncology

Question	Answer	Entry Date	Author
Does patient have an active cancer diagnosis?	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Discharge Planning (CM/SW)

Question	Answer	Entry Date	Author
Living Arrangements	Other	02/17/2024 11:58 AM PST	Your Scripps Health Team
Has discharge transport been arranged?	CM	02/17/2024 11:58 AM PST	Your Scripps Health Team
Transport company name and number	AMR WC shuttle	02/17/2024 11:58 AM PST	Your Scripps Health Team
What day is the transport expected?	66887	02/17/2024 11:58 AM PST	Your Scripps Health Team
What time is the transport expected?	50400	02/17/2024 11:58 AM PST	Your Scripps Health Team
Does the patient have someone to pick them up at discharge?	No	02/17/2024 11:58 AM PST	Your Scripps Health Team

Discharge Planning

Question	Answer	Entry Date	Author
Designated Family Caregiver	Documented in Emergency Contacts	01/19/2024 6:43 AM PST	Your Scripps Health Team
Home Care Services	No	01/24/2024 7:00 AM PST	Your Scripps Health Team
(Retired) Patient expects to be discharged to:	TBD	01/19/2024 6:43 AM PST	Your Scripps Health Team
Support Systems	Family members	01/24/2024 7:00 AM PST	Your Scripps Health Team
(Retired) Assistance Needed	with housing situation, pt states unsafe at this time	01/19/2024 6:43 AM PST	Your Scripps Health Team
Where was pt living / residing	Homeless, unsheltered	01/24/2024 7:00 AM PST	Your Scripps Health Team
Prior assistance needed	None/ Denies Need	01/24/2024 7:00 AM PST	Your Scripps Health Team
Patient expects to be discharged to	Not Known	01/24/2024 7:00 AM PST	Your Scripps Health Team

Confusion Assessment Method (ICU-CAM)

Question	Answer	Entry Date	Author
Feature 1: Acute Onset or Fluctuating Course	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team
Feature 2: Inattention	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team
Feature 4: Disorganized Thinking	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team
Overall CAM	Negative	02/13/2024 7:50 AM PST	Your Scripps Health Team

RUE Neurovascular Assessment

Question	Answer	Entry Date	Author
R Radial Pulse	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team

LUE Neurovascular Assessment

Question	Answer	Entry Date	Author
L Radial Pulse	+2	02/17/2024 7:15 AM PST	Your Scripps Health Team

RLE Neurovascular Assessment

Question	Answer	Entry Date	Author
R Posterior Tibial Pulse	+1	02/17/2024 7:15 AM PST	Your Scripps Health Team
R Dorsalis Pedis Pulse	Doppler	02/17/2024 7:15 AM PST	Your Scripps Health Team

LLE Neurovascular Assessment

Question	Answer	Entry Date	Author
L Posterior Tibial Pulse	+1	02/17/2024 7:15 AM PST	Your Scripps Health Team
L Dorsalis Pedis Pulse	Doppler	02/17/2024 7:15 AM PST	Your Scripps Health Team

Genitalia

Question	Answer	Entry Date	Author
Female Genitalia	Intact	02/16/2024 7:35 PM PST	Your Scripps Health Team

Genitourinary

Question	Answer	Entry Date	Author
Genitourinary Symptoms	None	02/17/2024 7:15 AM PST	Your Scripps Health Team
Genitourinary (WDL)	WDL	02/17/2024 7:15 AM PST	Your Scripps Health Team

Genitourinary Additional Assessments

Question	Answer	Entry Date	Author
Genitourinary Additional Assessments	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Cough and Deep Breathe

Question	Answer	Entry Date	Author
Cough and Deep Breathe	Yes	02/16/2024 7:35 PM PST	Your Scripps Health Team

Respiratory Interventions

Answer	Entry Date	Author
Cough and deep breathe	02/16/2024 7:35 PM PST	Your Scripps Health Team

Social History

Question	Answer	Entry Date	Author
Type of Residence	Shelter	02/12/2024 10:59 AM PST	Kalpana S

Musculoskeletal

Question	Answer	Entry Date	Author
Musculoskeletal (WDL)	X	02/16/2024 7:35 PM PST	Your Scripps Health Team
Traction in Use	No	02/16/2024 7:35 PM PST	Your Scripps Health Team
Musculoskeletal Additional Assessments	No	02/16/2024 7:35 PM PST	Your Scripps Health Team

Trauma/Abuse Screening

Question	Answer	Entry Date	Author
Physical Abuse	Yes, present (Comment)	01/19/2024 6:39 AM PST	Your Scripps Health Team
Verbal Abuse	Yes, present (Comment)	01/19/2024 6:39 AM PST	Your Scripps Health Team
Sexual Abuse	Denies	01/19/2024 6:39 AM PST	Your Scripps Health Team

Values/Beliefs

Question	Answer	Entry Date	Author
Cultural Practices During Hospitalization	none	01/19/2024 6:47 AM PST	Your Scripps Health Team
Spiritual Practices During Hospitalization	catholic	01/19/2024 6:47 AM PST	Your Scripps Health Team

Cognition

Question	Answer	Entry Date	Author
Arousal/Alertness	Appropriate responses to stimuli	02/12/2024 10:59 AM PST	Kalpana S
Judgement	Good	02/12/2024 10:59 AM PST	Kalpana S
Overall Cognitive Status	Intact	02/12/2024 10:59 AM PST	Kalpana S

Intake

Question	Answer	Entry Date	Author
I.V.	10	02/17/2024 4:00 PM PST	Your Scripps Health Team

Neuro Assessment

Question	Answer	Entry Date	Author
Alertness	Intact	02/09/2024 10:05 AM PST	Megan G
Coordination	Impaired	01/21/2024 8:31 AM PST	Deborah S
Sensation	Intact	02/09/2024 10:05 AM PST	Megan G

Incentive Spirometry

Question	Answer	Entry Date	Author
Incentive Spirometry Achieved (mL)	500	02/09/2024 9:22 AM PST	Your Scripps Health Team
Incentive Spirometry Goal (mL)	2500	02/06/2024 9:00 PM PST	Your Scripps Health Team

Discharge Summaries - documented in this encounter

Krishen Wadhwani, DO - 02/17/2024 2:52 PM PST

Formatting of this note might be different from the original.

[image]

Scripps Inpatient Discharge Summary

Patient Name: Elizabeth Tina Petruccio
MR#: 810760324
Admission Date: 1/12/2024 **Discharge Date:** 2/18/2024

Admitting Provider: Christopher W Jones, MD

Discharge Provider: No att. providers found
**Consults Ordered: **IP CONSULT TO PROCEDURE TEAM
IP CONSULT TO VASCULAR ACCESS
IP CONSULT TO SOCIAL WORK
IP CONSULT TO SPIRITUAL CARE
IP CONSULT TO SOCIAL WORK
IP CONSULT TO PODIATRY
IP CONSULT TO ENT
IP CONSULT TO NUTRITION SERVICES
Primary Care Physician at Discharge: No primary care provider on file. None
[table#1]:
CHIEF COMPLAINT
Shortness of breath

[table#2]:
[table#2.1]

[table#2.1]:
DISCHARGE DIAGNOSES
Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)
Calculus of gallbladder without cholecystitis without obstruction
Biliary colic
Severe sepsis (CMS/HCC)

Unless specifically stated, all of the above were present on admission.

[table#3]:
HISTORY OF PRESENT ILLNESS
Elizabeth Tina Petruccio is a 59 y.o. female with past medical history of heart failure reduced ejection fraction on echo, reported history of asthma unclear if she had PFTs, medical noncompliance, past alcohol and methamphetamine abuse who was brought in from ambulance at the border. History obtained from discussion with ED admitting physician as on my exam patient not waking up sufficiently to corroborate history, does shake her head appropriately though while on BiPAP, knows what hospital and city she is in, and why she was admitted..

Per report, patient has been feeling unwell over the last week with cough fevers shortness of breath chest pain and pressure, had been visiting her son in Mexico for unclear duration. Normally gets her care in Long Beach California. She reportedly went to a hospital in Mexico but did not agree with the treatment there so was brought to the border where she then called an ambulance and was then brought to our hospital here. She was given oxygen for comfort, on admission to the ED she was placed on BiPAP for respiratory distress, as well as poor air movement per report. She was on a BiPAP and diltiazem for A-fib with RVR, given complexity ICU was consulted for admission.

[table#4]:
HOSPITAL COURSE BY PROBLEM

 # Cervical discitis/osteomyelitis C4-C6, improving
 # Retropharyngeal and prevertebral fluid collection
During patient's hospital course, patient was complaining of persistent neck pain. CT of the neck demonstrated infection of the cervical disks/osteomyelitis. Infectious disease was consulted, recommending 8 weeks of Rocephin and Flagyl. ENT was consulted, which recommended repeat imaging. Suspicion is likely that there was bacterial translocation from mouth to retropharyngeal space and subsequently cervical spine. Neurosurgery was also consulted, who recommended repeat evaluation upon outpatient with either neurosurgery or orthopedic surgery. Patient will need to continue IV antibiotics until 3/21.

#Acute hypoxic respiratory failure, due to empyema
#Severe sepsis, secondary to pneumococcal pneumonia
#Right-sided empyema
Patient initially admitted to the ICU, during which time a chest tube was placed. Patient was also placed on supplemental oxygen, however she failed and was subsequently intubated. Patient subsequently extubated. Patient strep pneumonia antigen was positive on 1/14. Patient's empyema completely resolved, and patient currently off of oxygen.

#Biliary colic
#Cholelithiasis, without evidence of acute cholecystitis
Patient complaining of persiste
nt right upper quadrant pain, but underwent RUQ ultrasound early during admission. Negative for evidence of acute cholecystitis. Repeat right upper quadrant ultrasound close for 2 discharge showed gallstones and biliary sludge, no signs of acute cholecystitis. Patient underwent HIDA scan, which was found to be normal. Patient will need to follow-up with outpatient PCP for general surgery referral if patient is continuing to have biliary colic.

#Atrial fibrillation with RVR
Likely due to patient's sepsis above, was on amiodarone and diltiazem gtt.; however, this was discontinued. Patient's CHA2DS2-VASc or was 2 points, however with discussion, atrial fibrillation may be paroxysmal. Which shared decision making, it was elected that patient start Eliquis 5 mg twice daily

#Heart failure reduced ejection fraction, not in acute exacerbation
Patient resumed on Coreg, Entresto, and Farxiga upon resolution of severe sepsis above. Patient was resumed on oral Lasix.

#Euvolemic, hypotonic hyponatremia, likely due to SIADH from lung pathology above
Patient was placed on fluid restriction. Patient sodium subsequently normalized.

#Hypertension
Patient continued on carvedilol 3.125 mg twice daily and Entresto. Follow-up outpatient PCP for further titration.

#Contrast-induced AKI
Patient's creatinine was noted to be 1.22, following administration of contrast. Patient's Lasix was held, no fluids were given, with subsequent improvement of creatinine.

[table#5]:
DIAGNOSTIC TESTS/PROCEDURES

12-LEAD ECG
12-LEAD ECG
12-LEAD ECG
12-LEAD ECG
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL
ULTRASOUND ABDOMEN LIMITED LIVER
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
X-RAY CHEST SINGLE FRONTAL VIEW
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
CT CHEST WITHOUT CONTRAST
VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
X-RAY CHEST SINGLE FRONTAL VIEW
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
X-RAY FOOT RIGHT 3 OR MORE VIEWS
X-RAY CHEST SINGLE FRONTAL VIEW
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
X-RAY CHEST SINGLE FRONTAL VIEW
MRI BRAIN WITHOUT CONTRAST
MRI CERVICAL SPINE WITHOUT CONTRAST
MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST
X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL
CT CERVICAL SPINE WITH CONTRAST
VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL
IR PICC LINE PLACEMENT - US/FLUORO GUIDANCE AS NEEDED
X-RAY ANKLE RIGHT 3 OR MORE VIEWS
X-RAY ANKLE LEFT 3 OR MORE VIEWS
VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL
X-RAY HIP RIGHT 2-3 VIEWS
ULTRASOUND ABDOMEN LIMITED GALLBLADDER
NUC MED HEPATOBILIARY SYSTEM WITHOUT PHARMACOLOGY
ECHOCARDIOGRAM 2D WITH IMAGE ENHANCEMENT AGENT AS NEEDED

CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)

| --- |

[table#7]:
Pending Labs

[table#8]:
	Order	Current Status
	Extra Tubes	In process
	Extra Tubes	In process
	Extra Tubes	In process
	Extra Tubes	In process
	Extra Tubes	In process
	Extra Tubes	In process
	Extra Tubes	In process
	Extra Tubes	In process
	Extra Tubes	In process
	Extra Tubes	In process
	Gold Top	In process
	Gold Top	In process
	Gold Top	In process
	Lavender Top	In process
	Lavender Top	In process
	Lavender Top	In process
	Lavender Top	In process
	Lavender Top	In process
	Lavender Top	In process
	Light Green Top	In process
	Light Green Top	In process
	Light Green Top	In process
	Light Green Top	In process
	12-Lead ECG	Preliminary result
	AFB Culture w/Stain	Preliminary result

[table#9]:

[table#10]:
PHYSICAL EXAM AT DISCHARGE
Temp: [37 °C (98.6 °F)] 37 °C (98.6 °F)
Heart Rate: [97] 97
Resp: [18] 18
BP: (125)/(78) 125/78
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+1 pitting edema around ankles and bilateral feet, R > L
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular- difficult to elicit pulses i
n bilateral lower extremities.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#11]:
DISCHARGE CONDITION & DISPOSITION
Improved
Skilled nursing facility
[table#12]:
DISCHARGE MEDICATIONS

[table#13]:
| |

| --- |
| **Discharge Medications** |
| |

[table#14]:
Scheduled

[table#15]:
	Dose & Frequency
Advair HFA 115-21 mcg/actuation inhaler
Generic drug: fluticasone propion-salmeteroL
2 puffs, inhalation, 2 times daily
ammonium lactate 12 % lotion
Commonly known as: LAC-HYDRIN
topical, 2 times daily
apixaban 5 mg tablet
Commonly known as: ELIQUIS
5 mg, oral, 2 times daily
aspirin 81 mg tablet
81 mg, oral, Daily, Caloptima Mcal ID 93931022D30310
carvediloL 3.125 mg tablet
Commonly known as: COREG
3.125 mg, oral, 2 times daily, Caloptima Mcal, ID 93931022D30310
cefTRIAXone 1 gram injection
Commonly known as: ROCEPHIN
2 g, intraVENOUS, Every 24 hours
dapagliflozin propanediol 10 mg tablet tablet
Commonly known as: FARXIGA
5 mg, oral, Daily
furosemide 40 mg tablet
Commonly known as: LASIX
40 mg, oral, Daily
gabapentin 100 mg capsule
Commonly known as: NEURONTIN
200 mg, oral, 3 times daily
***** **lidocaine-menthol** 3.6-1.25 % patch
Commonly known as: LIDOPATCH
2 patches, transdermal, Every 24 hours
***** **lidocaine-menthol** 3.6-1.25 % patch
Commonly known as: LIDOPATCH
1 patch, transdermal, Every 24 hours
***** **lidocaine-menthol** 3.6-1.25 % patch
Commonly known as: LIDOPATCH
2 patches, transdermal, Every 24 hours
multivit-iron-FA-calcium-mins 9 mg iron-400 mcg tablet
Commonly known as: Thera M Plus
1 tablet, oral, Daily
potassium chloride 10 mEq CR capsule
Commonly known as: MICRO-K
10 mEq, oral, Daily
sacubitriL-valsartan 24-26 mg tablet
Commonly known as: ENTRESTO
1 tablet, oral, 2 times daily

[table#16]:
[table#16.1]

[table#16.1]:
[image:very important] *** There are duplicate medications prescribed to the patient**
[table#17]:

[table#18]:
PRN

[table#19]:
	Dose & Frequency
albuterol HFA 90 mcg/actuation inhaler
Commonly known as: PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA
2 puffs, inhalation, Every 4 hours PRN, Caloptima Mcal ID 93931022D30310

[table#20]:

[table#21]:
Pharmacy Information

[table#22]:
These medications were sent to Walgreens #05771 - HUNTINGTON BEACH, CA - 17522 BEACH BLVD AT SEC OF BEACH & SLATER	17522 BEACH BLVD, HUNTINGTON BEACH CA 92647-6802

[table#23]:
	Hours: 24-hours	Phone: 714-596-5272
- furosemide 40 mg tablet
[table#24]:

[table#25]:
Information on where to get these meds is not yet available.
Ask your nurse or doctor about these medications
- ammonium lactate 12 % lotion
- apixaban 5 mg tablet
- cefTRIAXone 1 gram injection
- dapagliflozin propanediol 10 mg tablet tablet
- gabapentin 100 mg capsule
- lidocaine-menthol 3.6-1.25 % patch
- lidocaine-menthol 3.6-1.25 % patch
- lidocaine-menthol 3.6-1.25 % patch
- multivit-iron-FA-calcium-mins 9 mg iron-400 mcg tablet
- sacubitriL-valsartan 24-26 mg tablet
[table#26]:

[table#27]:
DISCHARGE INSTRUCTIONS

?
Follow-Up Instructions
[table#28]:
	Follow up with Wadhwani, Krishen, DO in 2 week(s)
	Phone: 619-662-4100
	Where: San Ysidro Health

[table#29]:

[table#30]:
Contact information for after-discharge care

[table#31]:
	[table#31.1]

[table#31.1]:

[table#32]:
| | [table#32.1]
[table#32.2]
[table#32.3] |
| --- | --- |

[table#32.1]:
Destination

[table#32.2]:
	Mission Hills Post Acute Care
	Phone: 619-297-4484
	Fax: 619-297-4490
	Where: 3680 REYNARD WAY, SAN DIEGO CA 92103-3847
	Service: Skilled Nursing

[table#32.3]:

[table#33]:

PT/OT/SLT Recommendations
[table#34]:
PT/OT/SLP Recommendations

[table#35]:
Flowsheet Row	Most Recent Value
PT Dispo Recommendations	Consult with CM/SW for Placement filed at 02/09/2024 1005
PT Assistive Device Needs	4WW [image] [with a seat] filed at 02/09/2024 1005
PT Safety Concerns	fall risk without AD filed at 02/09/2024 1005
OT Dispo Recommendations	Consult with CM/SW for Placement filed at 02/12/2024 1059
OT Assistive Device Needs	FWW - front wheeled walker filed at 02/02/2024 1507
OT Safety Concerns	homelss, pain in R foot filed at 02/02/2024 1507
SLP Safety Concerns	Aspiration Precautions filed at 01/15/2024 1022
SLP Discharge Summary	Dysphagia 3/all liquids filed at 01/15/2024 1022
SLP Discharge Dispo Recommendation	Home filed at 01/15/2024 1022

[table#36]:

Advance Directive/POLST:
An entry from prior encounter but no active order

My total discharging and care coordination time was > 30 minutes
Electronically signed by: Krishen Wadhwani, DO, 2/18/2024 2:59 PM

Cosigned by Martha Strauser, MD at 02/22/2024 9:58 AM PST
Electronically signed by Krishen Wadhwani, DO at 02/18/2024 3:14 PM PST
Electronically signed by Krishen Wadhwani, DO at 02/18/2024 3:15 PM PST
Electronically signed by Martha Strauser, MD at 02/19/2024 12:32 PM PST
Electronically signed by Krishen Wadhwani, DO at 02/21/2024 4:46 PM PST
Electronically signed by Martha Strauser, MD at 02/22/2024 9:58 AM PST

Discharge Instructions - documented in this encounter

Discharge Instructions

Krishen Wadhwani, DO - 01/15/2024 3:40 PM PST
Formatting of this note is different from the original.
Images from the original note were not included.
Social Work Resources:

SAN DIEGO RESOURCES
Access and Crisis Line (888) 724-7240
Info Line: 211 or 858-300-1211

SHELTERS:
Name Population Served Address Phone Accommodations
Father Joe's Villages San Diego Adults with children 1501 Imperial Avenue, SD 619.233.8500 Call 211 for bed availability
The Salvation Army Single adult 18+ 825 7th Avenue 619.699.2218 Homeless outreach: 8:30-10:30a M-Th
VOA Luhman Center Shelter Program Adults with mental health disorders 290 S. Magnolia Ave El Cajon, CA 619.447.2428
San Diego Rescue Mission Nueva Vida Haven Emergency Overnight Shelter Women with children, 17 yo or younger 120 Elm Street, San Diego, CA 92101 619. 819-1844 Intake hours are 5:30pm- 7:30pm every day of the week
South Bay Community Services DV shelter, limited for homeless families Confidential 800.640.2933
619.420.3620 Intake 24hrs/day by phone/walk-in
PATH Connections Interim Housing

Adults 18+
1250 6th Ave
San Diego, CA 92101

619.810.8600

Call 211 for bed availability

Rachel's Women Shelter Women 18+ 759 8th Avenue
San Diego, CA 92101 619.696.0873 Call M-F
Day Center for availability

Intakes in person only for Night Shelter from 9am

Alpha Project
Bridge Shelter Single men & women 16th Street and Newton
17th and Commerical 858.568.8648

858. 354.1397 Call 211 for bed availability
Alpha Project -Casa Raphael Transitional Housing Program, Substance Abuse Recovery & Employment

Men Only

Provides 9-12 Month Residential Substance Abuse Treatment and Transitional 993 Postal Way (Near Escondido Ave and S. Santa Fe Ave); Vista, CA 92083 760.630-9922 Call To Set Up Intake Appointment; Intakes Monday Through Friday 9:00AM-5:00PM

Housing For Homeless Single Men

Casa Nueva Vida

Couples, Families with Children, Single Women/Men with children

430 F St, Chula Vista, CA 91910

619.420.3620

Call, Referral, Walk-In for services 24hrs/7 days a week

ISN Rotational Shelter Men, women, families with children Various sites 619.702.5399 Seasonal emergency shelter
Interfaith Shelter Network, San Diego Beaches Branch

Couples, Families with children, and Single Adults;

Location Rotates Bi-Weekly

619.231.2828 opt 1

619.692.2077 x108

Call and leave a VM with name, phone#, # of people in household
8a-6p
Mon-Thurs

Interfaith Shelter Network, South Bay Branch

Couples, Families with children, and Single Adults

Location Rotates Bi-Weekly

619.495.9151

Call for appointment, Intake Mon-Thurs 9AM-7PM, Fri 9AM-5PM

Interfaith Shelter Network, San Diego Inland Branch

Couples, Families with children, and Single Adults
Location Rotates Bi-Weekly
619.231.2828 opt 2

Call for appointment 9a-12p and 1p-4p Mon-Thurs

Interfaith North County- Haven House Shelter Year round shelter for adults 18 years and older

Breakfast and dinner provided

Must be drug/Alcohol free 550 W. Washington Ave., Escondido 760.536.3880 Call after 4 PM to be pre-screened; 760-489-6380 x274

Interfaith North County -Operation Hope Shelter
https://www.operationhopeshelter.org/

45 bed year round shelter - families and single women

Non married couples welcome -
Dinner/Breakfast

Must be drug/Alcohol free 859 East Vista Way Vista CA 92084 760.536.3880
Interfaith Community Services- Veteran's Bridge Housing and Transitional Housing

Shelter to transitional housing program for veterans

Must be drug/Alcohol free

1617 Mission Avenue, Oceanside, CA 92058

760.529.9979

La Posada de Guadalupe Shelter https://www.friendsoflaposada.org/

Year round shelter for 18yo up men and farm workers

Must be drug/Alcohol free 2476 Impala Dr., Carlsbad, CA 92101 760.929.2322 Call for availability; no walk ins

SOLUTIONS FOR CHANGE
Long term, 18yrs+, call to request an application.

***Not an emergency shelter

Must be drug/Alcohol free 722 W California Avenue, Vista CA 760.941.6545
Overnight Shelter for Homeless Transition Age Youth

Youth only (ages 18-25);

3090 Polk Ave
San Diego, CA 92104

619.415.6616

Walk-In For Services 5:30p-7:30p Tues and some Fridays or Call

Dreams for Change
Safe Parking Program

Homeless in cars

766 28th St, SD; 5605 Imperial Ave, San Diego, CA 92114

619.497.0236

619.356.1270

Call for availability/waitlist
Intake Mon-Sun 6p to 830pm

Overnight stay from 6p daily.

Leave before 6am at 28th street address; leave before 8am at Imperial ave address.
Jewish Family Services Safe Parking Encinitas, Balboa Ave., Aero Dr. and Mission Valley 858-637-3373
Sober Living Homes Association www.soberhousing.net/san-diego-sober-living.html

Men or Women 858.483.5866 Need a Source Of Income To Cover Minimum Housing Costs (From $350/Month)

OTHER RESOURCES
Name Population Served Address Phone Accommodations
Community Resource Center
650 Second Street, Encinitas, CA 92024 760.753-8300 Mon-Thurs 8:00am-5:00pm, Fri 8am-1:30 pm

Food Pantry (Mon-Thurs 11 am-2:30 pm,
Fri 11 am-1:30 pm),

Bread Line (M-F 9-10:30 am) monthly food bags, case management, domestic violence shelter, crisis intervention, counseling

Part of the inter-shelter network (ISN) for winter; can call to get screened

VI-SPDAT screening done on walk in basis (Mon-Thurs 11 am-3 pm)
Brother Bennos
Clothing, Food, Transportation, Replacing Identification or Driver's License, Mail/Telephone Services, Case Management, Legal Assistance, Alcohol & Drug Rehabilitation Referrals 3260 Production Avenue (Off Airport Road); Oceanside, CA 92054 760.439.1244
M-F, 6:30am - 8:00am, Breakfast Is Available;
All Services End At 11:00am Saturday, Open 8am-1pm 10am - 11:30am, Brunch; Haircuts No Other Services Are Available; Closed Sun & Holidays

Bread of Life Rescue Mission
https://bolrescue.org/

Drug & Alcohol test required 1919 Apple St., Ste. M, Oceanside, CA 92054 760.722.0800
Food pantry M-W&F 9am-11:45am, provide boxes of food on a first come first serve basis

Dinner served nightly Sun-F, 6:00-6:30 pm. Bread of Life Dinner Card required
Family Health Centers of San Diego www.fhcsd.org
Various locations, services and programs throughout the county Please call 619.515.2300 for more information on which services are available at individual sites. Healthcare for the homeless program: please call 619.515.2551

TrueCare (formerly known as North County Health Services) Various locations, services and programs throughout the North County Please call 760.736.6767 for more information on which services are available at individual sites.
Some services include: Adult medicine, physical therapy, alcohol and drug services, counseling/mental health services, dental care, HIV services, pharmacy, vision care, women's medicine, pediatric medicine, insurance application assistance and patient care support services.

Nearby sites:
-TrueCare Encinitas, 1130 2nd Street, Encinitas, CA 92024
-TrueCare Carlsbad, 1295 Carlsbad Village Drive, Suite 100 Carlsbad, CA 92008
-TrueCare Oceanside, 605 Crouch St., Building C, Oceanside, CA 92054.
South Bay Guidance Wellness Recovery Center Walk-In Clinic, Mental Health 1196 3rd Ave, Chula Vista, CA 91911 619.427.4661
Mon/Wed/Fri 9a-5p
Thurs 12p-8p

VI-SPDAT (Vulnerability Index-Service Prioritization Decision Assistance Tool)
Assessment for Housing Assistance
Complete the Vulnerability Index-Service Prioritization Decision Assistance Tool at a number of sites to determine level of need, appropriate housing intervention and available housing resources within the Coordinated Entry System (CES).
Please call 2-1-1 if you need resource information and assessment site locations.
Apha Project- Casa Raphael
993 POSTAL WAY
VISTA, CA 92083
(Near Escondido Ave and S. Santa Fe Ave)
12pm-3pm, Mon, Tues, Wed, Fri 760-630-9922
Call to Set Up Intake Appointment; Intakes Monday Through Friday 9am-5pm Provides 9-12 Month Residential Substance Abuse Treatment and Transitional Housing For Homeless Single Men.
(Men Only - Transitional Housing Program, Substance Abuse Recovery & Employment) Community Resource Center
650 Second Street Encinitas, CA 92024 (760) 753-8300
VI-SPDAT screening done on walk in basis (Mon-Thurs 11am-3 pm)
Interfaith Community Services
550 W Washingto Ave. ESCONDIDO, CA 92025, (760) 489-6380 Ext 262.

HOMELESS SERVICES:

Homeless Outreach
SD HOT Team: 619-446-1010
Father Joe's: 619-213-9409
Alpha Project: 619-318-4441
PATH: 619-810-8600 Homeless Response Center
1401 Imperial Ave
San Diego, CA 92101
619-880-8810
Adult housing/homeless, Legal, Mental Health, transitional age youth,Veterans affairs services.
Think Dignity - The Transitional Storage Center
252 16th St., San Diego, CA
619-537-8736
Showers & Lockers M-F 7AM-11AM, 430p-730PM, Sat 8a-12p, Closed on Sundays.
Catholic Charities
619-231-2828
Food distribution and resources; homeless/housing services; immigration/refugee services.
Regional Task force on homelessness
858-292-7627

Courage to Call
211 or (858) 300-1211; 711 if you are hearing-impaired and ask to be connected to 2-1-1. This is a free, confidential, helpline staffed by veterans and family members that provides 24/7 peer advocacy to active-duty military personnel, veterans, reservists, guard members, and their families. Services include: Assistance to supportive housing for homeless veterans and families; Referrals to long-term mental health services; Assistance with VA Benefits and claims; Assistance with food resources.

PATH Supportive Services for Veteran Families
619-860-7108
Veterans Village of San Diego
619.393.2000; 619.393-2039
Will need DD214; Emergency housing; transitional and permanent housing; mental health and substance abuse services.

HOUSING ASSISTANCE
Please call 211 for resource information
Call 2-1-1 or 858-300-1211 and ask to be connected to an outreach specialist who will confirm eligibility and facilitate a referral through the City's Coordinated Shelter Intake Program.

Father Joe's - Day Center (18+)
299 17th St, San Diego 92101
(619) 230-7390
Mon-Fri, Show up at 730a for intake
Father Joe's - Village (families)
1501 Imperial Ave, San Diego 92101
(619) 233-8500 **Bring ID
Mon-Fri, 8a-430p
Rachel's Women's Center (single women, no families)
759 8th Ave., San Diego 92101
Night Shelter for women.
Arrive by 8am for intake.

South Bay Community Services (families)*
430 F St., Chula Vista, 91910 619-420-3620
Mon-Thur 830a-8p
Fri 830a-6p
(First FOUR!)
*Single Adults also welcome to come for resources
South Bay Community Services TAY Academy
(Ages 18-24)
746 Ada St.
Chula Vista, CA 91911
Orientation 2nd Wednesday of the month at 3:00pm
Office Hours: 830a- 5pm
*LGBTQ drop in center for ages 21 and under, Mon-Sun 12p-8p
CARE Community Center
12 N. Euclid Ave, National City, 91950
619-356-4489
**Call for an appointment in advance Mon-Thurs 9a-6p and Fri 9a-1pm

CRISIS HOUSES
Name Population Served Address Phone
Jary Barreto Mental health support 2865 Logan Avenue (in San Diego) Phone 619.232.4357
Halcyon Center Mental health support 1664 Broadway (in El Cajon) Phone 619.579.8685
Del Sur (bilingual) Mental health support 892 27th Street (in San Diego) Phone 619.575.4687
New Vistas Mental health support 734 10th Avenue (in San Diego) Phone 619.239.4663
Vista Balboa (handicap access) Mental health support 545 Laurel Street (in San Diego) Phone 619.233.4399
Jackson House
Mental health support 5332 Jackson Dr (La Mesa) Phone 619.303.0933
Esperanza
(Single Floor) Mental health support 490 N. Grape Street (Escondido) Phone 760.975.9939
Turning Point Mental health support 1738 S. Tremont Street (Oceanside) Phone 760.439.2800

OUTPATIENT PSYCHIATRY CLINICS
Call Access and Crisis line (888) 724-7240 to identify an available outpatient psychiatry clinic near you.
Urgent Walk in Services.
* Jane Westin Center 619.235.2600 M-F 8am to 6pm
* Southeast Mental Health 619.595.4400 M-F 9am-11:30am; Mon,Tues,Thurs, Fri 1pm to 3:30pm
* NHA Project Enable 619-266-2111 M-F 10am to 2pm
* Alianza Wellness Center 619-822-1800 M-F 8:30am to 4:30pm

* Douglas Young Clinic 858.695.2211
* South Bay Guidance Center 619.427.4661 * El Cajon County Clinic 619.401.5500
* St. Vincent de Paul 619.233.8500 x1421 * Heartland 619.440.5133
* Areta Crowell Center 619.233.3432 * Project Enable 619.263.6155

Exodus Recovery Walk-In, Mental Health Assessment Clinic 524 W. Vista Way, Vista, CA 92083 760.758-1150 Walk-In Hours:

9am-430pm Mon-Fri
No Appointments, Just Walk-In
Exodus Recovery Walk-In, Mental Health Assessment Clinic 1520 S. Escondido Blvd, Escondido, CA 92025 760.871.2020 Walk-In Hours: 9am-4pm, Mon-Fri
No Appointments, Just Walk-In
Maria Sardinas Wellness Recovery Center Walk-In Clinic, Mental Health 1465 30th St, Suite K, San Diego, CA 92154 619.428.1000

619.428.1091 fax Mon/Wed 9a-8p
Tue/Thur/Fri 9a-5p
North Coastal mental Health Clinic
www.mhsinc.org Call for appointment
1701 Misison Ave Suite 230
Oceanside, CA 92058 760-712-3535

North Inland Mental Health Clinic
www.mhsinc.org
Call for appointment
125 West Misison Ave Suite 103
Escondido, CA 92025 760-747-3424
Call for appointment

MEAL CENTERS

Name Address Phone Notes
So Others May Eat Inc. 1215 Mariners Way.

9222 Chesapeake Dr. 858.900.1275 BBQ Lunch: Tue 12-1pm
St. Vincent de Paul Village 1501 Imperial Avenue 619.233.8500 Lunch: M-Sat 11am - 12:30pm; Sun 9:30-11:30am
First Presbyterian Church 320 Date Street. 619.232.7513 Wednesdays 5pm
Sundays 2pm
Saturdays 9am-11:00am - food distribution in the court yard
The Salvation Army 825 7th Ave.

648 3rd Ave.
Chula Vista, CA 619.699.2215

619.422.7027 Food pantry on MWF 9am-12pm

*Only at Chula Vista location: 1st mon/month dinner 5pm
Urban Angels 825 7th Ave. 858.848.1098 Wed dinner 5:30-7pm
Brother Benno 3260 Production Ave, Oceanside, CA 760.439.1244 Breakfast & Sack Lunch: M-F 6:30-12:00am
Wesley Community Services Center 5380 El Cajon Blvd. 619.354.8094 Breakfast MWF 8:30am-10pm; Lunch MF 1030am; Dinner W 4pm
New Day Urban Ministries 2459 Market Street 619.232.2753 Mon-Fri 8:30am - 11am
Mon-Thur 1pm-2:30pm
Bayside Community Center - call first 2202 Comstock Street (Linda Vista) 858.278.0771 Wednesdays 1pm-3pm
Food distribution in the parking lot
Uptown Faith Community Service 3725 30th Street 619.542.9344 Mon-Fri 10am - 1:45pm (COVID hours)

Super Food Pantry Mon & Fri
3pm-6pm
Bread of Life Rescue Mission
1919 Apple St., Ste. M Oceanside, CA 92054

https://bolrescue.org/ 760.722.0800 Food pantry M-W&F 9am-11:45am, provide boxes of food on a first come first serve basis

Dinner served nightly Sun-F, 6:00-6:30 pm. Bread of Life Dinner Card required. (Drug & Alcohol test required).

Community Resource Center
650 Second Street Encinitas, CA 92024 760.753.8300 Mon-Thurs 8am-5pm
Fri 8am-130pm

Food Pantry (M-Thurs 11 am-2:30 pm, Fri 11 am-1:30 pm), Bread Line (M-F 9-10:30 am), monthly food bags, case management, domestic violence shelter, crisis intervention, counseling

Part of the inter-shelter network (ISN) for winter; can call to get screened

VI-SPDAT screening done on walk in basis (Mon-Thurs 11 am-3 pm)

CLOTHING
Name Address Phone Notes
New Day Urban Ministries 2459 Market Street 619.232.2753 Mon-Fri 8:30am - 11am
Mon-Thur 1pm-2:30pm
Uptown Faith Community Service 3725 30th Street 619.542.9344 Mon-Fri 10am - 1:45pm (COVID hours)

Super Food Pantry Mon & Fri
3pm-6pm
St. Vincent de Paul 1501 Imperial Avenue 619.233.8500 Mon-Sat 830-10 am, ID or social security card required
Brother Bennos
3260 Production Avenue (Off Airport Road); 760.439.1244
M-F, 6:30am - 8:00am, Breakfast Is Available;
All Services End At 11:00am Saturday, Open 8am-1pm 10am - 11:30am, Brunch; Haircuts No Other Services Are Available; Closed Sun & Holidays

DRUG AND ALCOHOL TREATMENT
* Volunteers of America / Call for location / 619.232.9343
* CRASH / outpatient treatment 619.263.6663 / residential treatment 619.233.8054
* McCallister East / detox and outpatient / 619.465.7303
* San Diego Rescue Mission / residential treatment for men / 619.819.1715
* San Diego Rescue Mission / residential treatment for women and children / 619.819.1715
* Salvation Army / residential treatment / 619.239.4037
* Home Avenue Clinic / heroin detox / Methadone & Subutex / 3940 Home Avenue / 619.262.8000
* Renaissance Center / detox, residential / 619.791.2730
* Sharp McDonald Center / detox, inpatient, outpatient / 858.637.6920

12-STEP HOTLINES
* Alcoholics Anonymous - 619.265.8762 * Narcotics Anonymous - 619.584.1007
* AA North County - 760.758.2514 * Al-Anon (families) - 619.296.6666
* Alcoholicos Anonimos (en Espanol) - 619.280.7224 * Co-Dependents Anon. - 619.222.1244
* Cocaine Anonymous - 858.268.9109

Discharge instructions:

You are being discharged from the hospital after being found to have infections of your lungs causing fluid to build up and an infection of your cervical spine. Because of this, it is very important that you complete your course of IV antibiotics. This is specifically rocephin (ceftriaxone), which you will need daily for a total of 8 weeks (last day is on 3/21). You currently have a PICC line in, to receive these antibiotics but it is important to discontinue this once finished with antibiotics, as it can cause increased risk of infection and clotting.

Please make sure you follow up with your primary care physician 1-2 weeks following hospital discharge for further management of your other medical conditions. You will also need to follow up with an orthopedic or neurosurgeon for follow up on your cervical spine infection for possible re-imaging and further surgical management.

If you beign to experience symptoms of worsening chest pain, shortness of breath, or fever/chills, please return to the ER for further evaluation.
Electronically signed by Your Scripps Health Team at 01/15/2024 3:40 PM PST
Electronically signed by Krishen Wadhwani, DO at 02/16/2024 3:58 PM PST
Electronically signed by Krishen Wadhwani, DO at 02/16/2024 4:00 PM PST
Electronically signed by Krishen Wadhwani, DO at 02/16/2024 4:02 PM PST
Electronically signed by Krishen Wadhwani, DO at 02/16/2024 4:04 PM PST
Electronically signed by Krishen Wadhwani, DO at 02/16/2024 4:06 PM PST

Medications at Time of Discharge - documented as of this encounter

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
Advair HFA 115-21 mcg/ actuation inhaler	Inhale 2 puffs 2 (two) times a day.					
ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.				02/16/2024	
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			02/17/2024	
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch	Place 2 patches on the skin every 24 (twenty-four) hours.	60 patch			02/17/2024	03/18/2024
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch	Place 1 patch on the skin every 24 (twenty-four) hours.	30 patch			02/17/2024	03/18/2024
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch	Place 2 patches on the skin every 24 (twenty-four) hours.	60 patch			02/17/2024	03/18/2024
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g	3		12/23/2021	09/18/2024
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation.	60 tablet			02/16/2024	08/14/2024
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	09/18/2024
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day. Caloptima Mcal, ID 93931022D30310	60 tablet			12/23/2021	09/18/2024
cefTRIAXone (ROCEPHIN) 1 gram injection	Infuse 20 mL (2 g total) into a venous catheter every 24 (twenty-four) hours for 34 doses Indications: Osteomyelitis, discitis.	680 mL			02/17/2024	03/14/2024
dapagliflozin propanediol (FARXIGA) 10 mg tablet tablet	Take 0.5 tablets (5 mg total) by mouth daily.	45 tablet			02/17/2024	09/18/2024
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	30 tablet			02/16/2024	08/14/2024
gabapentin (NEURONTIN) 100 mg capsule	Take 2 capsules (200 mg total) by mouth 3 (three) times a day.	180 capsule			02/16/2024	03/08/2024
potassium chloride (MICRO-K) 10 mEq CR capsule	Take 10 mEq by mouth daily.				11/04/2023	08/14/2024
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	60 tablet			02/16/2024	08/14/2024

Ordered Prescriptions - documented in this encounter

Prescription	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			02/17/2024	
ammonium lactate (LAC-HYDRIN) 12 % lotion	Apply topically 2 (two) times a day.				02/16/2024	
sacubitriL-valsartan (ENTRESTO) 24-26 mg tablet	Take 1 tablet by mouth 2 (two) times a day.	60 tablet			02/16/2024	08/14/2024
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch	Place 2 patches on the skin every 24 (twenty-four) hours.	60 patch			02/17/2024	03/18/2024
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch	Place 1 patch on the skin every 24 (twenty-four) hours.	30 patch			02/17/2024	03/18/2024
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch	Place 2 patches on the skin every 24 (twenty-four) hours.	60 patch			02/17/2024	03/18/2024
gabapentin (NEURONTIN) 100 mg capsule	Take 2 capsules (200 mg total) by mouth 3 (three) times a day.	180 capsule			02/16/2024	03/08/2024
dapagliflozin propanediol (FARXIGA) 10 mg tablet tablet	Take 0.5 tablets (5 mg total) by mouth daily.	45 tablet			02/17/2024	09/18/2024
cefTRIAXone (ROCEPHIN) 1 gram injection	Infuse 20 mL (2 g total) into a venous catheter every 24 (twenty-four) hours for 34 doses Indications: Osteomyelitis, discitis.	680 mL			02/17/2024	03/14/2024
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet (5 mg total) by mouth 2 (two) times a day Indications: Non-Valvular Atrial Fibrillation.	60 tablet			02/16/2024	08/14/2024
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily.	30 tablet			02/16/2024	08/14/2024

Progress Notes - documented in this encounter

Stephanie J - 02/17/2024 12:29 PM PST

Formatting of this note might be different from the original.

SW Closing Note

Patient is medically cleared for transfer to SNF for IV antibiotics. CCM able to place patient in an alternative bed at Mission Hills Post Acute. Transport scheduled for today at 2pm. SNF will assist with discharge plan to a shelter or housing program upon completion of treatment. No additional needs identified at time of discharge.

Plan: No further SW intervention indicated.

Stephanie J., LCSW 2/17/24

Electronically signed by Stephanie J at 02/17/2024 12:31 PM PST

Your Scripps Health Team - 02/17/2024 12:04 PM PST

Formatting of this note might be different from the original.

[table#1]:
	02/17/24 1158
Final Discharge Planning
Living Arrangements	Other
(unsheltered)
Patient/Healthcare Decision Maker Notified of DC	Yes
Patient/Health Care Decision Maker Agrees to DC Plan	Yes
Designated Family Caregiver Notified of Discharge	Yes
Patient Choice List Given	Skilled Nursing Facility
Does the patient have someone to pick them up at discharge?	No
Has discharge transport been arranged?	CM
Transport company name and number	AMR WC shuttle
What day is the transport expected?	02/17/24
What time is the transport expected?	1400
DC Coordination Finalized	Yes
Outpatient Dialysis Information
Does patient receive outpatient dialysis treatment?	No
**Facility Contact **
Sending Case Manager	George Nye RN
Sending Physician	Dr. Strauser
Sending Nursing Unit	4th
Sending Nursing Unit Phone Number	6196917370
Receiving Physician Name	Dr. Bhavsar
Receiving Physician Phone Number	6195770917
Interpreter Services
Communication Needs	None

PASRR completed and sent by CM yesterday via the portal file transfer.
Patient is in agreement with the DC plan.
George E. Nye, RN, 2/17/2024, 12:05 PM

Electronically signed by Your Scripps Health Team at 02/17/2024 12:05 PM PST

Martha Strauser, MD - 02/17/2024 9:55 AM PST

Formatting of this note might be different from the original.

****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
Known to me 55-year-old homeless female with history of heart failure with reduced ejection fraction of 25 to 35%, alcohol abuse, asthma, methamphetamine use disorder with negative tox screen on admission.. Admitted on January 12, 2024 with shortness of breath. Found to have a large pleural effusion with atelectasis and empyema. S cervical spine tatus post chest tube placement. Status post intubation due to acute hypoxic hypercarbic respiratory failure. During ICU stay developed A-fib with rapid ventricular response. Patient was extubated her chest tube was removed on January 21, 2024 after treatment with tPA DNase. Later on developed neck pain. Had an MRI of cervical spine which reveals cervical discitis/osteomyelitis from C4-C6 with retropharyngeal and prevertebral fluid collection. Evaluated by neurosurgery not recommending any procedure at the present time. Consulted by

ID. Is to continue ceftriaxone for at least 8 weeks in conjunction with metronidazole for 2 weeks. Has been evaluated by ENT due to prevertebral fluid collection and they recommended follow-up with CAT scans. Was evaluated by IR who recommended no drainage. Status post PICC line on January 30. Has been on Rocephin since January 25. Is to complete antibiotics on March 22. Neck pain improved on gabapentin. Was complaining of leg swelling. Restarted on Lasix. Doing better.
C/o rib pain. USG with sludge and known GS. Has had 3 episodes of pain during this hospitalization. HIDA scan with normal gallbladder and no evidence of biliary duct dilation.

Objective
[table#2]:
**OBJECTIVE **
Temp: 37.3 °C (99.1 °F)
Heart Rate: 98
Resp: 20
BP: 123/74
SpO2: 95 %
Temp (24hrs), Avg:37.1 °C (98.7 °F), Min:36.8 °C (98.2 °F), Max:37.3 °C (99.1 °F)

Intake/Output Summary (Last 24 hours) at 2/17/2024 0955
Last data filed at 2/17/2024 0721
[table#3]:
Gross per 24 hour
Intake	760 ml
Output	900 ml
Net	**-140 ml**

SpO2: 95 %
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI -
no organomegaly appreciated, SNTND
Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs
[table#4]:
Results from last 7 days
Lab	Units	02/16/24
0545	02/14/24
0547	02/13/24
0736
SODIUM	mmol/L	137	137	139	139
POTASSIUM	mmol/L	4.4	4.2	4.6	4.6
CHLORIDE	mmol/L	101	101	103	103
CO2	mmol/L	30	**33***	**31*	31***
BUN	mg/dL	**23***	**20***	**22*	22***
CREATININE	mg/dL	0.80	0.96	0.80	0.80
CALCIUM	mg/dL	9.1	8.9	9.1	9.1
BILIRUBIN TOTAL	mg/dL	**0.1***	--	**0.1***
ALK PHOS	Units/L	80	--	79
ALT	Units/L	26	--	25
AST	Units/L	**38***	--	35
GLUCOSE	mg/dL	113	104	95	95

[table#5]:
Results from last 7 days
Lab	Units	02/16/24
0545	02/13/24
0736
WBC	K/mcL	5.0	5.3
HGB	g/dL	**9.7***	**9.8***
HEMATOCRIT	%	**32.3***	**32.2***

PLT	K/mcL	305	333
NEUTROS PCT AUTO	%	38.2	44.1
LYMPHS PCT AUTO	%	45.0	40.9
MONOS PCT AUTO	%	8.2	8.9
EOS PCT AUTO	%	7.4	4.9

[table#6]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/28/2024 07:25 AM	130 (H)	70 - 125 mg/dL	Final
01/26/2024 12:06 PM	131 (H)	70 - 125 mg/dL	Final
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final

Imaging Studies
Nuc Med hepatobiliary system without pharmacology

Result Date: 2/16/2024
IMPRESSION: Normal gallbladder visualization. There is no evidence of acute cholecystitis or biliary ductal dilation. release

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
colchicine, 0.6 mg, oral, Daily
dapagliflozin propanediol, 5 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 40 mg, oral, Daily
gabapentin, 200 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current Infusion Medications:
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/12/24 1610
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/16/24 0054
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/08/24 0339
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE-acetaminophen, 1 tablet, Q4H PRN
oxyCODONE-acetaminophen, 1 tablet, Q4H PRN, 1 tablet at 02/17/24 0621
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/16/24 2304

Assessment/Plan
[table#7]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with history of alcohol and methamphetamine use disorder, dilated cardiomyopathy with ejection fraction of 25 to 35%. Admitted with shortness of breath and hypoxic hypercarbic respiratory failure requiring intubation. Found to have a right pleural effusion s/p chest tube treatment with tPA DNase. L
ater on developed neck pain found to have C4-C5 C5-C6 discitis osteomyelitis with prevertebral fluid collection which has been evaluated by ENT and IR. Per ID continue Rocephin to complete 8 weeks as well as metronidazole for 14 days.

-Cervical discitis with osteomyelitis affecting C4-C5 C5-C6-continue Rocephin (1/25/-3/22)to complete 8 weeks of treatment. Continue ENT and ID recommendations.
-Status post acute hypoxic hypercarbic respiratory failure resolved. Status post intubation and extubation on January 14.
-Sepsis secondary to pneumococcus with right-sided empyema. The status post chest tube placement from January 13 to January 21.
-Dilated cardiomyopathy with reduced ejection fraction 25 to 35%-continue carvedilol, Entresto, Lasix, good candidate for SGLT2 inhibitor.
-COPD-stable
-A-fib with rapid ventricular test follows with CHA2DS2-VASc 2 score of 2-patient decided for anticoagulation with Eliquis
-Weakness-PT OT
-Methamphetamine and tobacco abuse-resources in the community have been provided to patient
-Right ankle gout-colchicine
-Oral ulcers a status post Harrop's outbreak-pain much improved with acyclovir
-DVT prophylaxis Eliquis.
-Disposition-SNF
LOS: 36 days
**Disposition Plan: **Quarantined environment pending medical progression.

Code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 02/17/2024 11:18 AM PST

Your Scripps Health Team - 02/16/2024 2:51 PM PST

Formatting of this note might be different from the original.
If medically cleared, pt can dc to the alternative bed this weekend. F/u in Ecin with Mission Hills Post Acute once pt is medically cleared. See SW and long stay team notes as well confirming this.

CM to remain avail

PASSR Level 1 done by previous CM. PASRR CID 203-348-670 file exchanged successfully to mission hills post acute already

Brad K. 2/16/2024, 2:53 PM
RN, Case Manager Scripps Chula Vista Hospital
** **

**1600: AMR on will call for tomorrow, 2/17 for w/c shuttle. Pt is not BLS. **

Brad K. 2/16/2024, 4:02 PM
RN, Case Manager Scripps Chula Vista Hospital
** **

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**

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Electronically signed by Your Scripps Health Team at 02/16/2024 2:53 PM PST
Electronically signed by Your Scripps Health Team at 02/16/2024 4:02 PM PST
Electronically signed by Your Scripps Health Team at 02/16/2024 4:06 PM PST

Krishen Wadhwani, DO - 02/16/2024 1:38 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amo
unt of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/9/2024: Patient is doing well except her stable neck pain no neurologic deficits. Patient also complaining of RUQ pain overnight, obtained RUQ ultraslund. Currently awaiting SNF placement.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 111/79 (BP Location: Left arm, Patient Position: Lying) | Pulse 62 | Temp 36.6 °C (97.9 °F) (Oral) | Resp 19 | Ht 1.676 m (5' 6") | Wt 65.1 kg (143 lb 8.3 oz) | LMP (LMP Unknown) | SpO2 90% | Breastfeeding No | BMI 23.16 kg/m²
I/O: In: 1130 [P.O.:1120; I.V.:10]
Out: 2150 [Urine:2000; Stool:150]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+1 pitting edema around ankles and bilateral feet, R > L
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular- difficult to elicit pulses in bilateral lower extremities.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/16/24
0545	02/14/24
0547	02/13/24
0736
WBC	K/mcL	5.0	--	5.3
HGB	g/dL	**9.7***	--	**9.8***
MCV	fL	**101***	--	**101***
PLT	K/mcL	305	--	333
SODIUM	mmol/L	137	137	139	139
POTASSIUM	mmol/L	4.4	4.2	4.6	4.6
CHLORIDE	mmol/L	101	101	103	103
CO2	mmol/L	30	**33***	**31*	31***
BUN	mg/dL	**23***	**20***	**22*	22***
CREATININE	mg/dL	0.80	0.96	0.80	0.80
CALCIUM	mg/dL	9.1	8.9	9.1	9.1
PHOSPHORUS	mg/dL	--	**5.6***	**5.1***
GLUCOSE	mg/dL	113	104	95	95
ALBUMIN	g/dL	3.8	**3.4***	3.8	3.8
AST	Units/L	**38***	--	35
ALT	Units/L	26	--	25
BILIRUBIN TOTAL	mg/dL	**0.1***	--	**0.1***
ALK PHOS	Units/L	80	--	79

[table#5]:
Recent Results (from the past 720 hour(s))
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value

| | Blood Culture | No Growth Final |
| **Blood Culture, Peripheral #2** |
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

Ultrasound abdomen limited gallbladder

Result Date: 2/15/2024
IMPRESSION: 1. Tumefactive sludge favored over nonshadowing new gallstones compared to negative gallbladder study of 1/13/2024. 2. Slight gallbladder wall thickening without pericholecystic edema or sonographic Murphy sign not so pronounced to confirm acute cholecystitis. Nuclear medicine biliary scan can clarify if additional imaging indicated clinically. 3. Borderline hepatomegaly with slightly echogenic dense sonographic pattern suspicious for mild hepatic steatosis. No surface contour abnormality, or ascites to suggest cirrhosis. No visible lesion to suggest infiltrative malignancy. release

Assessment/Plan

[table#6]:
Assessment and Plan

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

#RUQ pain
RUQ ultrasound shows sludge and new gallstones
- Follow up HIDA scan
- Pain management

 # Cervical discitis/osteomyelitis C4-C6, improving
 # Retropharyngeal and prevertebral fluid collection
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid
- IR will not be able to drain the area.
- PICC placed 1/30. Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).
-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of palms and feet, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA WNL
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy
Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
\
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
 # Bilateral leg edema
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg, Entresto, Farxiga 5mg.
-IV Lasix x 3 doses (2/8-2/10). Resume oral Lasix afterwards.
-Strict I's and O's while on Lasix

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
- Fluid restriction to 2L daily
- Monitor serum sodium.

#Hypertension
-Carvedilol 3.125 mg BID
-Entresto daily

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
- Colchicine dc'ed 1/25/2024 (due to rising creatinine) and resumed 2/9/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Elevated creatinine- resolved
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Resume Lasix 02/01/2024. Held Lasix given increased diuresis and hyponatremia

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage. Medical is currently approved, but still pending SNF placement.

Krishen Wadhwani, DO
PGY-2, Internal Medicine

Cosigned by Martha Strauser, MD at 02/17/2024 9:55 AM PST
Electronically signed by Krishen Wadhwani, DO at 02/16/2024 1:40 PM PST
Electronically signed by Martha Strauser, MD at 02/17/2024 9:55 AM PST

Howie Que, DPM - 02/16/2024 1:24 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT inte
rval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/16/24 0922
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/16/24 0920
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/16/24 0920 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/16/24 0920
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/15/24 1608
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 0.6 mg at 02/16/24 0919
• dapagliflozin propanediol (FARXIGA) tablet 5 mg, 5 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 5 mg at 02/16/24 0920
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/16/24 1153
• furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 40 mg at 02/16/24 0920
• gabapentin (NEURONTIN) capsule 200 mg, 200 mg, oral, TID, Wadhwani, Krishen, DO, 200 mg at 02/16/24 0921
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/12/24 1610
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, ne
bulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/16/24 0054
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at

01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, de Four, Amanda R, MD, 1 patch at 02/16/24 0924
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch, 2 patch, transdermal, Q24H, Wadhwani, Krishen, DO, 2 patch at 02/16/24 0923
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch, 2 patch, transdermal, Q24H, Wadhwani, Krishen, DO, 2 patch at 02/16/24 1154
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/08/24 0339
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/06/24 1459 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/16/24 0919
• oxyCODONE-acetaminophen (PERCOCET) 10-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, Wadhwani, Krishen, DO
• oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, Wadhwani, Krishen, DO
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/16/24 0921
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/16/24 0227

Allergies
Lactose

Objective

Vital Signs
[table#1]:
Vitals:
	02/16/24 0725
BP:	111/79
Pulse:	62
Resp:	19
Temp:	36.6 °C (97.9 °F)
SpO2:	90%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine and abx therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/16/24 0545
SODIUM	mmol/L	137

POTASSIUM	mmol/L	4.4
CHLORIDE	mmol/L	101
CO2	mmol/L	30
BUN	mg/dL	**23***
CREATININE	mg/dL	0.80
GLUCOSE	mg/dL	113
CALCIUM	mg/dL	9.1

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute o
n chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/16/241:24 PM

Electronically signed by Howie Que, DPM at 02/16/2024 1:25 PM PST

Hai Shao, MD, PhD - 02/16/2024 1:13 PM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
No fever overnight, feeling well with no new complaints.

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
colchicine, 0.6 mg, oral, Daily
dapagliflozin propanediol, 5 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 40 mg, oral, Daily
gabapentin, 200 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/12/24 1610
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/16/24 0054
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/08/24 0339
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459

And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE-acetaminophen, 1 tablet, Q4H PRN
oxyCODONE-acetaminophen, 1 tablet, Q4H PRN
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/16/24 0227

Current Vital Signs:
[table#2]:
Vitals:
	02/15/24 2120	02/16/24 0054	02/16/24 0644	02/16/24 0725
BP:	121/88			111/79
BP Location:	Left arm			Left arm
Patient Position:	Sitting			Lying
Pulse:	103			62
Resp:	18			19
Temp:	36.9 °C (98.4 °F)			36.6 °C (97.9 °F)
TempSrc:	Oral			Oral
SpO2:	95%	93%		90%
Weight:			65.1 kg (143 lb 8.3 oz)	
Height:				

Physical Exam
Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No tenderness. No edema.
Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriente
d to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	02/16/24
0545 | 02/13/24

0736 | 02/10/24
0552 |
WBC	K/mcL	5.0	5.3	6.0
HGB	g/dL	**9.7***	**9.8***	**10.1***
HEMATOCRIT	%	**32.3***	**32.2***	**32.4***
PLT	K/mcL	305	333	377
NEUTROS PCT AUTO	%	38.2	44.1	45.9
LYMPHS PCT AUTO	%	45.0	40.9	41.3
MONOS PCT AUTO	%	8.2	8.9	8.2
EOS PCT AUTO	%	7.4	4.9	3.5

[table#4]:
Results from last 7 days
Lab	Units	02/16/24
0545	02/14/24
0547	02/13/24
0736	02/11/24
2009	02/10/24
0552
SODIUM	mmol/L	137	137	139	139	< >	**132***
POTASSIUM	mmol/L	4.4	4.2	4.6	4.6	< >	4.5
CHLORIDE	mmol/L	101	101	103	103	< >	101
CO2	mmol/L	30	**33***	**31*	31***	< >	**31***
BUN	mg/dL	**23***	**20***	**22*	22***	< >	**21***
CREATININE	mg/dL	0.80	0.96	0.80	0.80	< >	0.87
CALCIUM	mg/dL	9.1	8.9	9.1	9.1	< >	9.0
BILIRUBIN TOTAL	mg/dL	**0.1***	--	**0.1***	--	0.2
ALK PHOS	Units/L	80	--	79	--	80
ALT	Units/L	26	--	25	--	21
AST	Units/L	**38***	--	35	--	33
GLUCOSE	mg/dL	113	104	95	95	< >	94
< > = values in this interval not displayed.

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

IMAGING:
Ultrasound abdomen limited gallbladder

Result Date: 2/15/2024
IMPRESSION: 1. Tumefactive sludge favored over nonshadowing new gallstones compared to negative gallbladder study of 1/13/2024. 2. Slight gallbladder wall thickening without pericholecystic edema or sonographic Murphy sign not so pronounced to confirm acute cholecystitis. Nuclear medicine biliary scan can clarify if additional imaging indicated clinically. 3. Borderline hepatomegaly with slightly echogenic dense sonographic pattern suspicious for mild hepatic steatosis. No surface contour

abnormality, or ascites to suggest cirrhosis. No visible lesion to suggest infiltrative malignancy. release

X-ray hip right 2-3 views

Result Date: 2/12/2024
IMPRESSION: 1. Mild degenerative changes at right hip joint. No acute fracture detected. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 2/11/2024
IMPRESSION: No deep venous thrombosis detected in either leg. release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	At
rial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with osteophytes. CRP is normalized but ESR still grossly elevated despite improvement. D/w RN, continue rocephin (day 22/56). Picc line placement and discharge planning. Doubt cholecystitis as pt has no fever or leukocytosis while on broad-spectrum antibiotics.

Dr. Baran will cover the next 2 days on prn basis.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 02/16/2024 1:15 PM PST

Stephanie J - 02/16/2024 11:39 AM PST
Formatting of this note might be different from the original.
SW Progress Note

Patient remains admitted for long-term IV antibiotics, with approximately 5 weeks left. CCM Keturah indicated an alternative bed is available at Mission Hills Post Acute. Per previous SW notes, patient is expected to discharge to the community upon completion of treatment.

SW met with patient at bedside to inform of possible SNF placement. Patient presented as A&Ox4 and very talkative. She spoke about her efforts to get settled in San Diego including Medi-Cal, GR, and EBT. She is very interested in housing programs and had several questions about obtaining vouchers. SW discussed CES and accessing shelter system in the interim. Patient was agreeable to this, though mildly anxious due to unfamiliarity with the system.

SW assisted patient in accessing SNF website for her to look into their care. She ultimately stated agreement with this transfer. Patient endorsed some concerns for gallstones and MD already ordered work up. CCM indicated that patient would be appropriate to transfer over the weekend pending medical clearance. Patient verbalized understanding for the plan and had no additional needs at this time.

Plan: SW to continue to monitor for discharge planning.

Stephanie J., LCSW 2/16/24

Electronically signed by Stephanie J at 02/16/2024 11:57 AM PST

Your Scripps Health Team - 02/16/2024 10:55 AM PST

Formatting of this note might be different from the original.
Referral sent to MHPA for alternative SNF bed. Waiting to hear from SNF. Barriers: Chronic homelessness. History of ETOH and meth use, although UDS was negative on admission.

Update 1142: MHPA can accept. Updated Katie at MHPA that MD has ordered further diagnostics for gallbladder. SNF can accept Sat or Sun if pt is medically cleared. Floor CM can arrange with transfer directly with SNF.

Electronically signed by Your Scripps Health Team at 02/16/2024 10:57 AM PST
Electronically signed by Your Scripps Health Team at 02/16/2024 11:44 AM PST

Martha Strauser, MD - 02/16/2024 9:00 AM PST
Formatting of this note might be different from the original.
****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
Known to me 55-year-old homeless female with history of heart failure with reduced ejection fraction of 25 to 35%, alcohol abuse, asthma, methamphetamine use disorder with negative tox screen on admission.. Admitted on January 12, 2024 with shortness of breath. Found to have a large pleural effusion with atelectasis and empyema. S cervical spine tatus post chest tube placement. Status post intubation due to acute hypoxic hypercarbic respiratory failure. During ICU stay developed A-fib with rapid ventricular response. Patient was extubated her chest tube was removed on January 21, 2024 after treatment with tPA DNase. Later on developed neck pain. Had an MRI of cervical spine which reveals cervical discitis/osteomyelitis from C4-C6 with retropharyngeal and prevertebral fluid collection. Evaluated by neurosurgery not recommending any procedure at the present time. Consulted by ID. Is to continue ceftriaxone for at least 8 weeks in conjunction with metronidazole for 2 weeks. Has been evaluated by ENT due to prevertebral fluid collection and they recommended follow-up with CAT scans. Was evaluated by IR who recommended no drainage. Status post PICC line on January 30. Has been on Rocephin since January 25. Is to complete antibiotics on March 22. Neck pain improved on gabapentin. Was complaining of leg swelling. Restarted on Lasix. Doing better.
C/o rib pain. USG with sludge and known GS.
Has had 3 episodes of pain during this hospitalization. Will check HIDA scan.

Objective
[table#2]:
**OBJECTIVE **
Temp: 36.6 °C (97.9 °F)
Heart Rate: 62
Resp: 19
BP: 111/79
SpO2: 90 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 0 L/min
Temp (24hrs), Avg:36.8 °C (98.2 °F), Min:36.6 °C (97.9 °F), Max:36.9 °C (98.4 °F)

Intake/Output Summary (Last 24 hours) at 2/16/2024 0900
Last data filed at 2/16/2024 0500
[table#3]:
Gross per 24 hour
Intake	770 ml
Output	2150 ml
Net	**-1380 ml**

SpO2: 90 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 0 L/min
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND
Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs
[table#4]:
Results from last 7 days
Lab	Units	02/16/24
0545	02/14/24
0547	02/13/24
0736	02/11/24
2009	02/10/24
0552
SODIUM	mmol/L	137	137	139	139	< >	**132***
POTASSIUM	mmol/L	4.4	4.2	4.6	4.6	< >	4.5
CHLORIDE	mmol/L	101	101	103	103	< >	101
CO2	mmol/L	30	**33***	**31*	31***	< >	**31***
BUN	mg/dL	**23***	**20***	**22*	22***	< >	**21***
CREATININE	mg/dL	0.80	0.96	0.80	0.80	< >	0.87
CALCIUM	mg/dL	9.1	8.9	9.1	9.1	< >	9.0
BILIRUBIN TOTAL	mg/dL	**0.1***	--	**0.1***	--	0.2
ALK PHOS	Units/L	80	--	79	--	80
ALT	Units/L	26	--	25	--	21
AST	Units/L	**38***	--	35	--	33
GLUCOSE	mg/dL	113	104	95	95	< >	94
< > = values in this interval not displayed.

[table#5]:
Results from last 7 days
Lab	Units	02/16/24
0545	02/13/24
0736	02/10/24
0552
WBC	K/mcL	5.0	5.3	6.0
HGB	g/dL	**9.7***	**9.8***	**10.1***
HEMATOCRIT	%	**32.3***	**32.2***	**32.4***
PLT	K/mcL	305	333	377
NEUTROS PCT AUTO	%	38.2	44.1	45.9
LYMPHS PCT AUTO	%	45.0	40.9	41.3
MONOS PCT AUTO	%	8.2	8.9	8.2
EOS PCT AUTO	%	7.4	4.9	3.5

[table#6]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/28/2024 07:25 AM	130 (H)	70 - 125 mg/dL	Final
01/26/2024 12:06 PM	131 (H)	70 - 125 mg/dL	Final
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final

Imaging Studies
Ultrasound abdomen limited gallbladder

Result Date: 2/15/2024
IMPRESSION: 1. Tumefactive sludge favored over nonshadowing new gallstones compared to negative gallbladder study of 1/13/2024. 2. Slight gallbladder wall thickening without pericholecystic edema or sonographic Murphy sign not so pronounced to confirm acute cholecystitis. Nuclear medicine biliary scan can clarify if additional imaging indicated clinically. 3. Borderline hepatomegaly with slightly echogenic dense sonographic pattern suspicious for mild hepatic steatosis. No surface contour abnormality, or ascites to suggest cirrhosis. No visible lesion to suggest infiltrative malignancy. release

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H

colchicine, 0.6 mg, oral, Daily
dapagliflozin propanediol, 5 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 40 mg, oral, Daily
gabapentin, 200 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-va
lsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current Infusion Medications:
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.25 mg, Q4H PRN, 0.25 mg at 02/16/24 0457
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/12/24 1610
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/16/24 0054
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/08/24 0339
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE-acetaminophen, 1 tablet, Q4H PRN, 1 tablet at 02/15/24 2320
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/16/24 0227

Assessment/Plan
[table#7]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with history of alcohol and methamphetamine use disorder, dilated cardiomyopathy with ejection fraction of 25 to 35%. Admitted with shortness of breath and hypoxic hypercarbic respiratory failure requiring intubation. Found to have a right pleural effusion s/p chest tube treatment with tPA DNase. Later on developed neck pain found to have C4-C5 C5-C6 discitis osteomyelitis with prevertebral fluid collection which has been evaluated by ENT and IR. Per ID continue Rocephin to complete 8 weeks as well as metronidazole for 14 days.

-Cervical discitis with osteomyelitis affecting C4-C5 C5-C6-continue Rocephin (1/25/-3/22)to complete 8 weeks of treatment. Continue ENT and ID recommendations.
-Status post acute hypoxic hypercarbic respiratory failure resolved. Status post intubation and extubation on January 14.
-Sepsis secondary to pneumococcus with right-sided empyema. The status post chest tube placement from January 13 to January 21.
-Dilated cardiomyopathy with reduced ejection fraction 25 to 35%-continue carvedilol, Entresto, Lasix, good candidate for SGLT2 inhibitor.
-COPD-stable
-A-fib with rapid ventricular test follows with CHA2DS2-VASc 2 score of 2-patient decided for anticoagulation with Eliquis
-Weakness-PT OT
-Methamphetamine and tobacco abuse-resources in the community have been provided to patient
-Right ankle gout-colchicine
-Oral ulcers a status post Harrop's outbreak-pain much improved with acyclovir
-DVT prophylaxis Eliquis.
-Disposition-SNF
LOS: 35 days
**Disposition Plan: **Quarantined environment pending medical progression.

Code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 02/16/2024 12:02 PM PST

Howie Que, DPM - 02/15/2024 8:39 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improv
ed pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/15/24 0826
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/15/24 0825
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/15/24 0825 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/15/24 0825
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/15/24 1608
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 0.6 mg at 02/15/24 0824
• dapagliflozin propanediol (FARXIGA) tablet 5 mg, 5 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 5 mg at 02/15/24 0825
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/15/24 1631
• furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 40 mg at 02/15/24 0824
• gabapentin (NEURONTIN) capsule 200 mg, 200 mg, oral, TID, Wadhwani, Krishen, DO, 200 mg at 02/15/24 1608
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.25 mg, 0.25 mg, intraVENOUS, Q4H PRN, de Four, Amanda R, MD, 0.25 mg at 02/15/24 0156
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/12/24 1610
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/14/24 1759
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024

• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, de Four, Amanda R, MD, 1 patch at 02/15/24 0825
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch, 2 patch, transdermal, Q24H, Wadhwani, Krishen, DO, 2 patch at 02/15/24 0825
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch, 2 patch, transdermal, Q24H, Wadhwani, Krishen, DO, 2 patch at 02/14/24 1406
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/08/24 0339
• ondansetron (PF) (ZOFRAN) injec
tion 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/06/24 1459 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/15/24 0825
• oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, de Four, Amanda R, MD, 1 tablet at 02/15/24 1801
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/15/24 0825
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/14/24 2250

Allergies
Lactose

Objective

Vital Signs
[table#1]:
Vitals:
	02/15/24 1828
BP:	
Pulse:	
Resp:	
Temp:	
SpO2:	95%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine and abx therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/14/24 0547
SODIUM	mmol/L	137
POTASSIUM	mmol/L	4.2

CHLORIDE	mmol/L	101
CO2	mmol/L	**33***
BUN	mg/dL	**20***
CREATININE	mg/dL	0.96
GLUCOSE	mg/dL	104
CALCIUM	mg/dL	8.9

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/15/248:40 PM

Electronically signed by Howie Que, DPM at 02/15/2024 8:40 PM PST

Krishen Wadhwani, DO - 02/15/2024 4:20 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respira
tory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amount of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/9/2024: Patient is doing well except her stable neck pain no neurologic deficits. No new complaints today. Currently awaiting SNF placement.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 113/82 (BP Location: Right arm, Patient Position: Sitting) | Pulse 96 | Temp 36.7 °C (98.1 °F) (Oral) | Resp 18 | Ht 1.676 m (5' 6") | Wt 64.1 kg (141 lb 5 oz) | LMP (LMP Unknown) | SpO2 93% | Breastfeeding No | BMI 22.81 kg/m²
I/O: In: 730 [P.O.:720; I.V.:10]

Out: 2200 [Urine:1850; Stool:350]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+1 pitting edema around ankles and bilateral feet, R > L
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular- difficult to elicit pulses in bilateral lower extremities.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/14/24
0547	02/13/24
0736	02/11/24
2009	02/10/24
0552
WBC	K/mcL	--	5.3	--	6.0
HGB	g/dL	--	**9.8***	--	**10.1***
MCV	fL	--	**101***	--	100
PLT	K/mcL	--	333	--	377
SODIUM	mmol/L	137	139	139	< >	**132***
POTASSIUM	mmol/L	4.2	4.6	4.6	< >	4.5
CHLORIDE	mmol/L	101	103	103	< >	101
CO2	mmol/L	**33***	**31*	31***	< >	**31***
BUN	mg/dL	**20***	**22*	22***	< >	*
*21***
CREATININE	mg/dL	0.96	0.80	0.80	< >	0.87
CALCIUM	mg/dL	8.9	9.1	9.1	< >	9.0
PHOSPHORUS	mg/dL	**5.6***	**5.1***	< >	--
GLUCOSE	mg/dL	104	95	95	< >	94
ALBUMIN	g/dL	**3.4***	3.8	3.8	< >	3.6
AST	Units/L	--	35	--	33
ALT	Units/L	--	25	--	21
BILIRUBIN TOTAL	mg/dL	--	**0.1***	--	0.2
ALK PHOS	Units/L	--	79	--	80
< > = values in this interval not displayed.

[table#5]:
Recent Results (from the past 720 hour(s))
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM

	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#6]:
Assessment and Plan

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6, improving
 # Retropharyngeal and prevertebral fluid collection
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid
- IR will not be able to drain the area.
- PICC placed 1/30. Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin f
or 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).
-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of palms and feet, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA WNL
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy
Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
 # Bilateral leg edema
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg, Entresto, Farxiga 5mg.
-IV Lasix x 3 doses (2/8-2/10). Resume oral Lasix afterwards.
-Strict I's and O's while on Lasix

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
- Fluid restriction to 2L daily
- Monitor serum sodium.

#Hypertension
-Carvedilol 3.125 mg BID
-Entresto daily

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
- Colchicine dc'ed 1/25/2024 (due to rising creatinine) and resumed 2/9/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Elevated creatinine- resolved
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Resume Lasix 02/01/2024. Held Lasix given increased diuresis and hyponatremia

 # Housing instability
**# Subs
tance use disorder (methamphetamine)**
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage. Medical is currently approved, but still pending SNF placement.

Krishen Wadhwani, DO
PGY-2, Internal Medicine

Cosigned by Martha Strauser, MD at 02/16/2024 8:59 AM PST
Electronically signed by Krishen Wadhwani, DO at 02/15/2024 4:22 PM PST
Electronically signed by Martha Strauser, MD at 02/16/2024 8:59 AM PST

Hai Shao, MD, PhD - 02/15/2024 2:13 PM PST

Formatting of this note might be different from the original.
Progress Note

Subjective:
No fever overnight, feeling well with no new complaints.

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
colchicine, 0.6 mg, oral, Daily
dapagliflozin propanediol, 5 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 40 mg, oral, Daily
gabapentin, 200 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.25 mg, Q4H PRN, 0.25 mg at 02/15/24 0156
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/12/24 1610
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/14/24 1759
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/08/24 0339
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE-acetaminophen, 1 tablet, Q4H PRN, 1 tablet at 02/15/24 1005
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/14/24 2250

Current Vital Signs:
[table#2]:
Vitals:
	02/14/24 1953	02/15/24 0553	02/15/24 0716	02/15/24 1115
BP:	122/80		113/82	
BP Location:	Left arm		Right arm	
Patient Position:	Sitting		Sitting	
Pulse:	110		82	96
Resp:	16		19	18
Temp:	37.1 °C (98.8 °F)		36.7 °C (98.1 °F)	
TempSrc:	Oral		Oral	
SpO2:	94%		96%	93%
Weight:		64.1 kg (141 lb 5 oz)		
Height:				

Physical Exam
Vitals reviewed.

Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Muscul
oskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No tenderness. No edema.
Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	02/13/24
0736	02/10/24
0552
WBC	K/mcL	5.3	6.0
HGB	g/dL	**9.8***	**10.1***
HEMATOCRIT	%	**32.2***	**32.4***
PLT	K/mcL	333	377
NEUTROS PCT AUTO	%	44.1	45.9
LYMPHS PCT AUTO	%	40.9	41.3
MONOS PCT AUTO	%	8.9	8.2
EOS PCT AUTO	%	4.9	3.5

[table#4]:
Results from last 7 days
Lab	Units	02/14/24
0547	02/13/24
0736	02/12/24
0528	02/11/24
2009	02/10/24
0552
SODIUM	mmol/L	137	139	139	138	< >	**132***
POTASSIUM	mmol/L	4.2	4.6	4.6	4.5	< >	4.5
CHLORIDE	mmol/L	101	103	103	102	< >	101
CO2	mmol/L	**33***	**31*	31***	**31***	< >	**31***
BUN	mg/dL	**20***	**22*	22***	**21***	< >	**21***
CREATININE	mg/dL	0.96	0.80	0.80	0.81	< >	0.87
CALCIUM	mg/dL	8.9	9.1	9.1	9.0	< >	9.0

BILIRUBIN TOTAL	mg/dL	--	**0.1***	--	--	0.2
ALK PHOS	Units/L	--	79	--	--	80
ALT	Units/L	--	25	--	--	21
AST	Units/L	--	35	--	--	33
GLUCOSE	mg/dL	104	95	95	97	< >	94
< > = values in this interval not displayed.

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

IMAGING:
X-ray hip right 2-3 views

Result Date: 2/12/2024
IMPRESSION: 1. Mild degenerative changes at right hip joint. No acute fracture detected. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 2/11/2024
IMPRESSION: No deep venous thrombosis detected in either leg. release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	E
mpyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:

No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with osteophytes. CRP is normalized but ESR still grossly elevated despite improvement. D/w RN, continue rocephin (day 21/56). Picc line placement and discharge planning.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 02/15/2024 2:14 PM PST

Your Scripps Health Team - 02/15/2024 12:40 PM PST

Formatting of this note might be different from the original.

NUTRITION CARE CHECK NOTE:

Patient average PO intake 75-100% x 13 meals. Orders meals from Room Service.Weight currently 64.1 kg 2/15; overall stable. Current diet is adequate and appropriate:

[table#1]:
|
|
| --- |
| [table#1.1] |

[table#1.1]:
Dietary Orders
(From admission, onward)

[table#2]:
|
|
| --- |

[table#3]:
| | [table#3.1]
[table#3.2] |
| --- | --- |

[table#3.1]:
	Start			Ordered
	02/04/24 0904		[table#3.1.1]	02/04/24 0904

[table#3.1.1]:
Combination Diet Regular; 2000 mL Diet effective now
[table#3.1.1.1]

[table#3.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Fluid Restriction - Oral (mL/day):	2000 mL	

[table#3.2]:

[table#4]:
.

Allergies:
[table#5]:
Allergies
Allergen	Reactions
•	Lactose	Other (see comments)
		Patient having diarrhea; only fluid milk - all other lactose items ok

Last BM Date: 02/14/24 (per patient.), Stool Appearance: Unable to assess.
Labs reviewed.

Continue to monitor and offer nutrition interventions as needed.

Cindy K DTR
Dietetic Technician Registered
Clinical Nutrition
Tie Line: 312-7176

Electronically signed by Your Scripps Health Team at 02/15/2024 12:44 PM PST

Martha Strauser, MD - 02/15/2024 9:58 AM PST
Formatting of this note might be different from the original.
****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
Known to me 55-year-old homeless female with history of heart failure with reduced ejection fraction of 25 to 35%, alcohol abuse, asthma, methamphetamine use disorder with negative tox screen on admission.. Admitted on January 12, 2024 with shortness of breath. Found to have a large pleural effusion with atelectasis and empyema. S cervical spine tatus post chest tube placement. Status post intubation due to acute hypoxic hypercarbic respiratory failure. During ICU stay developed A-fib with rapid ventricular response. Patient was extubated her chest tube was removed on January 21, 2024 after treatment with tPA DNase. Later on developed neck pain. Had an MRI of cervical spine which reveals cervical discitis/osteomyelitis from C4-C6 with retropharyngeal and prevertebral fluid collection. Evaluated by neurosurgery not recommending any procedure at the present time. Consulted by ID. Is to continue ceftriaxone for at least 8 weeks in conjunction with metronidazole for 2 weeks. Has been evaluated by ENT due to prevertebral fluid collection and they recommended follow-up with CAT scans. Was evaluated by IR who recommended no drainage. Status post PICC line on January 30. Has been on Rocephin since January 25. Is to complete antibiotics on March 22. Neck pain improved on gabapentin. Was complaining of leg swelling. Restarted on Lasix. Doing better.

Objective
[table#2]:
**OBJECTIVE **
Temp: 36.7 °C (98.1 °F)
Heart Rate: 82
Resp: 19
BP: 113/82
SpO2: 96 %
O2 Flow Rate (L/min): 2 L/min
Temp (24hrs), Avg:36.9 °C (98.5 °F), Min:36.7 °C (98.1 °F), Max:37.1 °C (98.8 °F)

Intake/Output Summary (Last 24 hours) at 2/15/2024 0958
Last data filed at 2/15/2024 0900
[table#3]:
Gross per 24 hour
Intake	1090 ml
Output	2200 ml
Net	**-1110 ml**

SpO2: 96 %
O2 Flow Rate (L/min): 2 L/min

Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND
Neuro - normal
MSK - MAE

Skin - no rash, no CCE

Labs
[table#4]:
Results from last 7 days
Lab	Units	02/14/24
0547	02/13/24
0736	02/12/24
0528	02/11/24
2009	02/10/24
0552
SODIUM	mmol/L	137	139	139	138	< >	**132***
POTASSIUM	mmol/L	4.2	4.6	4.6	4.5	< >	4.5
CHLORIDE	mmol/L	101	103	103	102	< >	101
CO2	mmol/L	**33***	**31*	31***	**31***	< >	**31***
BUN	mg/dL	**20***	**22*	22***	**21***	< >	**21***
CREATININE	mg/dL	0.96	0.80	0.80	0.81	< >	0.87
CALCIUM	mg/dL	8.9	9.1	9.1	9.0	< >	9.0
BILIRUBIN TOTAL	mg/dL	--	**0.1***	--	--	0.2
ALK PHOS	Units/L	--	79	--	--	80
ALT	Units/L	--	25	--	--	21
AST	Units/L	--	35	--	--	33
GLUCOSE	mg/dL	104	95	95	97	< >	94
< > = values in this interval not displayed.

[table#5]:
Results from last 7 days
Lab	Units	02/13/24
0736	02/10/24
0552
WBC	K/mcL	5.3	6.0
HGB	g/dL	**9.8***	**10.1***
HEMATOCRIT	%	**32.2***	**32.4***
PLT	K/mcL	333	377
NEUTROS PCT AUTO	%	44.1	45.9
LYMPHS PCT AUTO	%	40.9	41.3
MONOS PCT AUTO	%	8.9	8.2
EOS PCT AUTO	%	4.9	3.5

[table#6]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/28/2024 07:25 AM	130 (H)	70 - 125 mg/dL	Final
01/26/2024 12:06 PM	131 (H)	70 - 125 mg/dL	Final
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final

Imaging Studies
No results found.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
colchicine, 0.6 mg, oral, Daily
dapagliflozin propanediol, 5 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 40 mg, oral, Daily
gabapentin, 200 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H

lidocaine-menthol, 2 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current Infusion Medications:
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.25 mg, Q4H PRN, 0.25 mg at 02/15/24 0156
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/12/24 1610
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/14/24 1759
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/08/24 0339
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE-acetaminophen, 1 tablet, Q4H PRN, 1 tablet at 02/15/24 0552
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/14/24 2250

Assessment/Plan
[table#7]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with history of alcohol and methamphetamine use disorder, dilated cardiomyopathy with ejection fraction of 25 to 35%. Admitted with shortness of breath and hypoxic hypercarbic respiratory failure requiring intubation. Found to have a right pleural effusion s/p chest tube treatment with tPA DNase. Later on developed neck pain found to have C4-C5 C5-C6 discitis osteomyelitis with prevertebral fluid collection which has been evaluated by ENT and IR. Per ID continue Rocephin to complete 8 weeks as well as metronidazole for 14 days.

-Cervical discitis with osteomyelitis affecting C4-C5 C5-C6-continue Rocephin (1/25/-3/22)to complete 8 weeks of treatment. Continue ENT and ID recommendations.
-Status post acute hypoxic hypercarbic respiratory failure resolved. Status post intubation and extubation on January 14.
-Sepsis secondary to pneumococcus with right-sided empyema. The status post chest tube placement from January 13 to January 21.
-Dilated cardiomyopathy with reduced ejection fraction 25 to 35%-continue carvedilol, Entresto, Lasix, good candidate for SGLT2 inhibitor.
-COPD-stable
-A-fib with rapid ventricular test follows with CHA2DS2-VASc 2 score of 2-patient decided for anticoagulation with Eliquis
-Weakness-PT OT
-Methamphetamine and tobacco abuse-resources in the community have been provided to patient
-Right ankle gout-colchicine
-Oral ulcers a status post Harrop's outbreak-pain much improved with acyclovir
-DVT prophylaxis Eliquis.
-Disposition-SNF
LOS: 34 days
**Disposition Plan: **Quarantined environment pending medical progression.

Code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 02/15/2024 11:32 AM PST

Your Scripps Health Team - 02/15/2024 7:52 AM PST

Formatting of this note might be different from the original.

[image:untitled image]

Case Management Chart Review/DCP:
** **
** **
**Dx/Plan: **Pt admitted for sepsis and empysema. Pt is currently pending placement for long term IV abx but underfunded. Abx end date is 3/22/2024.

**PLOF: **Ind, homeless. Normally lives in Huntington Beach CA
**
Funding: **Mcal - Per HARs, still active in Orange County

DCP: Evolving. Not a candidate for alternative bed. Pt ambulating 200ft. Pt is on Mcal call list. Cm reviewed past several days of referral list; no accepting snf.

**Barrier: **underfunded, homeless, no accepting SNF
** **
** **
Channy RN
Inpatient Care Manager
Scripps Mercy Hospital

Electronically signed by Your Scripps Health Team at 02/15/2024 7:53 AM PST
Electronically signed by Your Scripps Health Team at 02/15/2024 9:31 AM PST

Your Scripps Health Team - 02/14/2024 5:23 PM PST

Formatting of this note might be different from the original.

Case Management Chart Review/DCP:
** **
** **
**Dx/Plan: **Pt admitted for sepsis and empysema. Pt is currently pending placement for long term IV abx but underfunded. ABX end date 03/22/2024

**PLOF: **Ind, homeless. Normally lives in Huntington Beach CA
**
Funding: **Mcal. PCS assisting with HMO assignment

DCP: Evolving. Not a candidate for alternative bed due to homelessness and no funding. On admin call list

**Barrier: **underfunded

Patricia B, MSN, RN, PHN, DNPc
Care Manager

Electronically signed by Your Scripps Health Team at 02/14/2024 5:25 PM PST

Your Scripps Health Team - 02/14/2024 3:27 PM PST

Formatting of this note might be different from the original.

[table#1]:
	02/14/24 1500
Clinical Encounter Type
Visit Date	02/14/24
Visited With	Patient
Routine Visit	Follow-up
Consult	Patient care
Emotional/Spiritual Services
Assessment	Elizabeth received ashes on her forehead.
Services Provided	Prayer
Emotional/Spiritual Outcomes
Outcome	Spiritual services available.
Continue Visiting	As needed

Electronically signed by Your Scripps Health Team at 02/14/2024 3:27 PM PST

Krishen Wadhwani, DO - 02/14/2024 2:49 PM PST

Formatting of this note might be different from the original.

[image]

Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have emp
yema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amount of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/9/2024: Patient is doing well except her stable neck pain no neurologic deficits. No new complaints today. Currently awaiting SNF placement.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 108/74 (BP Location: Left arm, Patient Position: Sitting) | Pulse 103 | Temp 37 °C (98.6 °F) (Oral) | Resp 15 | Ht 1.676 m (5' 6") | Wt 60.8 kg (134 lb 0.6 oz) | LMP (LMP Unknown) | SpO2 95% | Breastfeeding No | BMI 21.63 kg/m²
I/O: In: 550 [P.O.:510; I.V.:40]
Out: 1050 [Urine:1050]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush

Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+1 pitting edema around ankles and bilateral feet, R > L
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular- difficult to elicit pulses in bilateral lower extremities.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/14/24
0547	02/13/24
0736	02/11/24
2009	02/10/24
0552
WBC	K/mcL	--	5.3	--	6.0
HGB	g/dL	--	**9.8***	--	**10.1***
MCV	fL	--	**101***	--	100
PLT	K/mcL	--	333	--	377
SODIUM	mmol/L	137	139	139	< >	**132***
POTASSIUM	mmol/L	4.2	4.6	4.6	< >	4.5
CHLORIDE	mmol/L	101	103	103	< >	101
CO2	mmol/L	**33***	**31*	31***	< >	**31***
BUN	mg/dL	**20***	**22*	22***	< >	**21***
CREATININE	mg/dL	0.96	0.80	0.80	< >	0.87
CALCIUM	mg/dL	8.9	9.1	9.1	< >	9.0
PH
OSPHORUS	mg/dL	**5.6***	**5.1***	< >	--
GLUCOSE	mg/dL	104	95	95	< >	94
ALBUMIN	g/dL	**3.4***	3.8	3.8	< >	3.6
AST	Units/L	--	35	--	33
ALT	Units/L	--	25	--	21
BILIRUBIN TOTAL	mg/dL	--	**0.1***	--	0.2
ALK PHOS	Units/L	--	79	--	80
< > = values in this interval not displayed.

[table#5]:
Recent Results (from the past 720 hour(s))
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value

| | Blood Culture | No Growth Final |
| **Blood Culture, Peripheral #2** |
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#6]:
Assessment and Plan

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6, improving
 # Retropharyngeal and prevertebral fluid collection
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid
- IR will not be able to drain the area.
- PICC placed 1/30. Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lu
ng reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).
-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of palms and feet, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA WNL
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy
Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.

-Eliquis 5 mg BID

 # HFrEF not in exacerbation
 # Bilateral leg edema
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg, Entresto, Farxiga 5mg.
-IV Lasix x 3 doses (2/8-2/10). Resume oral Lasix afterwards.
-Strict I's and O's while on Lasix

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
- Fluid restriction to 2L daily
- Monitor serum sodium.

#Hypertension
-Carvedilol 3.125 mg BID
-Entresto daily

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
- Colchicine dc'ed 1/25/2024 (due to rising creatinine) and resumed 2/9/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Elevated creatinine- resolved
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Resume Lasix 02/01/2024.
Held Lasix given increased diuresis and hyponatremia

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage. Medical is currently approved, but still pending SNF placement.

Krishen Wadhwani, DO
PGY-2, Internal Medicine

Cosigned by Martha Strauser, MD at 02/15/2024 9:58 AM PST
Electronically signed by Krishen Wadhwani, DO at 02/14/2024 2:49 PM PST
Electronically signed by Martha Strauser, MD at 02/15/2024 9:58 AM PST

Howie Que, DPM - 02/14/2024 12:59 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/14/24 0825
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/14/24 0824
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/14/24 0824 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/14/24 0825
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/13/24 1708
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 0.6 mg at 02/14/24 0825
• dapagliflozin propanediol (FARXIGA) tablet 5 mg, 5 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 5 mg at 02/14/24 0828
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/14/24 1222
• furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 40 mg at 02/14/24 0825
• gabapentin (NEURONTIN) capsule 200 mg, 200 mg, oral, TID, Wadhwani, Krishen, DO, 200 mg at 02/14/24 0824
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.25 mg, 0.25 mg, intraVENOUS, Q4H PRN, de Four, Amanda R, MD, 0.25 mg at 02/14/24 0250
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/12/24 1610
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/14/24 0053
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, de Four, Amanda R, MD, 1 patch at 02/13/24 1911
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Once, Wadhwani, Krishen, DO, 1 patch at 02/14/24 1010
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch, 2 patch, transdermal, Q24H, Wadhwani, Krishen, DO, 2 patch at 02/13/24 1910
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch, 2 patch, transdermal, Q24H, Wadhwani, Krishen, DO
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/08/24 0339
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/06/24 1459 **AND** metoCLOPramide

(REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/14/24 0824
• oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, de Four, Amanda R, MD, 1 tablet at 02/14/24 0918
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/14/24 0824
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/13/24 2230

Allergies
Lactose

Objective

Vital Signs
[table#1]:
Vitals:
	02/14/24 0731
BP:	108/74
Pulse:	103
Resp:	15
Temp:	37 °C (98.6 °F)
SpO2:	95%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possibl
e right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine and abx therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/14/24 0547
SODIUM	mmol/L	137
POTASSIUM	mmol/L	4.2
CHLORIDE	mmol/L	101
CO2	mmol/L	**33***
BUN	mg/dL	**20***
CREATININE	mg/dL	0.96
GLUCOSE	mg/dL	104
CALCIUM	mg/dL	8.9

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/14/2412:59 PM

Electronically signed by Howie Que, DPM at 02/14/2024 1:00 PM PST

Hai Shao, MD, PhD - 02/14/2024 10:49 AM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
Events reviewed. No fever overnight, feeling well with no new complaints.

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
colchicine, 0.6 mg, oral, Daily
dapagliflozin propanediol, 5 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 40 mg, oral, Daily
gabapentin, 200 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Once
lidocaine-menthol, 2 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.25 mg, Q4H PRN, 0.25 mg at 02/14/24 0250
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/12/24 1610
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/14/24 0053
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/08/24 0339
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE-acetaminophen, 1 tablet, Q4H PRN, 1 tablet at 02/14/24 0918
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/13/24 2230

Current Vital Signs:

[table#2]:
Vitals:
	02/13/24 2000	02/14/24 0053	02/14/24 0500	02/14/24 0731
BP:	110/77			108/74
BP Location:	Left arm			Left arm
Patient Position:	Lying			Sitting
Pulse:	116	103		103
Resp:	18	18		15
Temp:	36.9 °C (98.4 °F)			37 °C (98.6 °F)
TempSrc:	Oral			Oral
SpO2:	94%	94%		95%
Weight:			60.8 kg (134 lb 0.6 oz)	
Height:				

Physical Exam
Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normoceph
alic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No tenderness. No edema.
Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	02/13/24
0736	02/10/24
0552
WBC	K/mcL	5.3	6.0
HGB	g/dL	**9.8***	**10.1***
HEMATOCRIT	%	**32.2***	**32.4***
PLT	K/mcL	333	377
NEUTROS PCT AUTO	%	44.1	45.9
LYMPHS PCT AUTO	%	40.9	41.3
MONOS PCT AUTO	%	8.9	8.2
EOS PCT AUTO	%	4.9	3.5

[table#4]:
Results from last 7 days
Lab	Units	02/14/24
0547	02/13/24
0736	02/12/24
0528	02/11/24
2009	02/10/24
0552
SODIUM	mmol/L	137	139	139	138	< >	**132***
POTASSIUM	mmol/L	4.2	4.6	4.6	4.5	< >	4.5
CHLORIDE	mmol/L	101	103	103	102	< >	101
CO2	mmol/L	**33***	**31*	31***	**31***	< >	**31***
BUN	mg/dL	**20***	**22*	22***	**21***	< >	**21***
CREATININE	mg/dL	0.96	0.80	0.80	0.81	< >	0.87
CALCIUM	mg/dL	8.9	9.1	9.1	9.0	< >	9.0
BILIRUBIN TOTAL	mg/dL	--	**0.1***	--	--	0.2
ALK PHOS	Units/L	--	79	--	--	80
ALT	Units/L	--	25	--	--	21
AST	Units/L	--	35	--	--	33
GLUCOSE	mg/dL	104	95	95	97	< >	94
< > = values in this interval not displayed.

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

IMAGING:
X-ray hip right 2-3 views

Result Date: 2/12/2024
IMPRESSION: 1. Mild degenerative changes at right hi
p joint. No acute fracture detected. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 2/11/2024
IMPRESSION: No deep venous thrombosis detected in either leg. release

X-ray ankle right 3 or more views

Result Date: 2/7/2024
IMPRESSION: 1. Marked soft tissue swelling at the ankle and hindfoot and midfoot and forefoot 2. Small plantar calcaneal spur 3. No fractures or dislocations identified release

X-ray ankle left 3 or more views

Result Date: 2/7/2024
IMPRESSION: 1. Severe soft tissue swelling around the ankle midfoot and forefoot correlate clinically for cellulitis. No radiographic evidence of osteomyelitis. Midfoot degenerative changes 2. No soft tissue emphysema release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with osteophytes. CRP is better but ESR still grossly elevated. D/w RN, continue rocephin (day 20/56). Picc line placement and discharge planning. Repeat CRP and ESR today.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 02/14/2024 10:50 AM PST

Martha Strauser, MD - 02/14/2024 8:54 AM PST
Formatting of this note might be different from the original.
****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
Known to me 55-year-old homeless female with history of heart failure with reduced ejection fraction of 25 to 35%, alcohol abuse, asthma, methamphetamine use disorder with negative tox screen on admission.. Admitted on January 12, 2024 with shortness of breath. Found to have a large pleural effusion with atelectasis and empyema. S cervical spine tatus post chest tube placement. Status post intubation due to acute hypoxic hypercarbic respiratory failure. During ICU stay developed A-fib with rapid ventricular response. Patient was extubated her chest tube was removed on January 21, 2024 after treatment with tPA DNase. Later on developed neck pain. Had an MRI of cervical spine which reveals cervical discitis/osteomyelitis from C4-C6 with retropharyngeal and prevertebral fluid collection. Evaluated by neurosurgery not recommending any procedure at the present time. Consulted by ID. Is to continue ceftriaxone for at least 8 weeks in conjunction with metronidazole for 2 weeks. Has been evaluated by ENT due to prevertebral fluid collection and they recommended follow-up with CAT scans. Was evaluated by IR who recommended no drainage. Status post PICC line on January 30. Has been on Rocephin since January 25. Is to complete antibiotics on March 22. Neck pain improved on gabapentin. Was complaining of leg swelling. Restarted on Lasix. Doing better.

Objective
[table#2]:
**OBJECTIVE **
Temp: 37 °C (98.6 °F)
Heart Rate: 103
Resp: 15
BP: 108/74
SpO2: 95 %

Temp (24hrs), Avg:36.9 °C (98.5 °F), Min:36.9 °C (98.4 °F), Max:37 °C (98.6 °F)

Intake/Output Summary (Last 24 hours) at 2/14/2024 0854
Last data filed at 2/14/2024 0200
[table#3]:
Gross per 24 hour
Intake	550 ml
Output	800 ml
Net	**-250 ml**

SpO2: 95 %
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND
Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs
[table#4]:
Results from last 7 days
Lab	U
nits	02/14/24
0547	02/13/24
0736	02/12/24
0528	02/11/24
2009	02/10/24
0552
SODIUM	mmol/L	137	139	139	138	< >	**132***
POTASSIUM	mmol/L	4.2	4.6	4.6	4.5	< >	4.5
CHLORIDE	mmol/L	101	103	103	102	< >	101
CO2	mmol/L	**33***	**31*	31***	**31***	< >	**31***
BUN	mg/dL	**20***	**22*	22***	**21***	< >	**21***
CREATININE	mg/dL	0.96	0.80	0.80	0.81	< >	0.87
CALCIUM	mg/dL	8.9	9.1	9.1	9.0	< >	9.0
BILIRUBIN TOTAL	mg/dL	--	**0.1***	--	--	0.2
ALK PHOS	Units/L	--	79	--	--	80
ALT	Units/L	--	25	--	--	21
AST	Units/L	--	35	--	--	33
GLUCOSE	mg/dL	104	95	95	97	< >	94
< > = values in this interval not displayed.

[table#5]:
Results from last 7 days
Lab	Units	02/13/24
0736	02/10/24
0552
WBC	K/mcL	5.3	6.0
HGB	g/dL	**9.8***	**10.1***
HEMATOCRIT	%	**32.2***	**32.4***
PLT	K/mcL	333	377
NEUTROS PCT AUTO	%	44.1	45.9
LYMPHS PCT AUTO	%	40.9	41.3
MONOS PCT AUTO	%	8.9	8.2
EOS PCT AUTO	%	4.9	3.5

[table#6]:
Glucose Blood, POC

Date/Time	Value	Ref Range	Status
01/28/2024 07:25 AM	130 (H)	70 - 125 mg/dL	Final
01/26/2024 12:06 PM	131 (H)	70 - 125 mg/dL	Final
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final

Imaging Studies
No results found.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
colchicine, 0.6 mg, oral, Daily
dapagliflozin propanediol, 5 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 40 mg, oral, Daily
gabapentin, 200 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current Infusion Medications:
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.25 mg, Q4H PRN, 0.25 mg at 02/14/24 0250
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/12/24 1610
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/14/24 0053
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/08/24 0339
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE-acetaminophen, 1 tablet, Q4H PRN, 1 tablet at 02/14/24 0452
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/13/24 2230

Assessment/Plan
[table#7]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with history of alcohol and methamphetamine use disorder, dilated cardiomyopathy with ejection fraction of 25 to 35%. Admitted with shortness of breath and hypoxic hypercarbic respiratory failure requiring intubation. Found to have a right pleural effusion s/p chest tube treatment with tPA DNase. Later on developed neck pain found to have C4-C5 C5-C6 discitis osteomyelitis with prever
tebral fluid collection which has been evaluated by ENT and IR. Per ID continue Rocephin to complete 8 weeks as well as metronidazole for 14 days.

-Cervical discitis with osteomyelitis affecting C4-C5 C5-C6-continue Rocephin (1/25/-3/22)to complete 8 weeks of treatment. Continue ENT and ID recommendations.

-Status post acute hypoxic hypercarbic respiratory failure resolved. Status post intubation and extubation on January 14.
-Sepsis secondary to pneumococcus with right-sided empyema. The status post chest tube placement from January 13 to January 21.
-Dilated cardiomyopathy with reduced ejection fraction 25 to 35%-continue carvedilol, Entresto, Lasix, good candidate for SGLT2 inhibitor.
-COPD-stable
-A-fib with rapid ventricular test follows with CHA2DS2-VASc 2 score of 2-patient decided for anticoagulation with Eliquis
-Weakness-PT OT
-Methamphetamine and tobacco abuse-resources in the community have been provided to patient
-Right ankle gout-colchicine
-Oral ulcers a status post Harrop's outbreak-pain much improved with acyclovir
-DVT prophylaxis Eliquis.
-Disposition-SNF
LOS: 33 days
**Disposition Plan: **Quarantined environment pending medical progression.

Code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 02/14/2024 11:30 AM PST

Howie Que, DPM - 02/13/2024 2:20 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/13/24 0908
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/13/24 0906
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/13/24 0907 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulos
e (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/13/24 0908

• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/12/24 1546
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 0.6 mg at 02/13/24 0907
• dapagliflozin propanediol (FARXIGA) tablet 5 mg, 5 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 5 mg at 02/13/24 0906
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/13/24 1203
• furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 40 mg at 02/13/24 0906
• gabapentin (NEURONTIN) capsule 200 mg, 200 mg, oral, TID, Wadhwani, Krishen, DO, 200 mg at 02/13/24 1412
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.25 mg, 0.25 mg, intraVENOUS, Q4H PRN, de Four, Amanda R, MD, 0.25 mg at 02/13/24 0224
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/12/24 1610
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/12/24 2019
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, de Four, Amanda R, MD
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch, 2 patch, transdermal, Q24H, Wadhwani, Krishen, DO, 2 patch at 02/12/24 1924
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/08/24 0339
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/06/24 1459 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/13/24 0906
• oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, de Four, Amanda R, MD, 1 tablet at 02/13/24 1413
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/13/24 0908
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/12/24 2205

Allergies
Lactose

Objective

Vital Signs
[table#1]:
Vitals:
	02/13/24 0726
BP:	**(!) 145/98**
Pulse:	123
Resp:	18
Temp:	36.7 °C (98.1 °F)
SpO2:	93%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitati
on/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:

1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine and abx therapy.

[table#2]:

Results from last 7 days
Lab	Units	02/13/24 0736
SODIUM	mmol/L	139	139
POTASSIUM	mmol/L	4.6	4.6
CHLORIDE	mmol/L	103	103
CO2	mmol/L	**31*	31***
BUN	mg/dL	**22*	22***
CREATININE	mg/dL	0.80	0.80
GLUCOSE	mg/dL	95	95
CALCIUM	mg/dL	9.1	9.1

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/13/242:20 PM

Electronically signed by Howie Que, DPM at 02/13/2024 2:21 PM PST

Krishen Wadhwani, DO - 02/13/2024 2:06 PM PST

Formatting of this note might be different from the original.

[image]

Hospital Medicine Progress Note

Subjective

[table#1]:

Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amount of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no sym
ptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/9/2024: Patient is doing well except her stable neck pain no neurologic deficits. No new complaints today. Currently awaiting SNF placement.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP (!) 145/98 (BP Location: Left arm, Patient Position: Sitting) | Pulse 123 | Temp 36.7 °C (98.1 °F) (Oral) | Resp 18 | Ht 1.676 m (5' 6") | Wt 60.1 kg (132 lb 7.9 oz) | LMP (LMP Unknown) | SpO2 93% | Breastfeeding No | BMI 21.39 kg/m²
I/O: In: 930 [P.O.:930]
Out: 700 [Urine:700]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+1 pitting edema around ankles and bilateral feet, R > L
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular- difficult to elicit pulses in bilateral lower extremities.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/13/24
0736	02/12/24
0528	02/11/24
2009	02/10/24
0552
WBC	K/mcL	5.3	--	--	6.0
HGB	g/dL	**9.8***	--	--	**10.1***
MCV	fL	**101***	--	--	100
PLT	K/mcL	333	--	--	377
SODIUM	mmol/L	139	139	138	< >	**132***
POTASSIUM	mmol/L	4.6	4.6	4.5	< >	4.5
CHLORIDE	mmol/L	103	103	102	< >	101
CO2	mmol/L	**31*	31***	**31***	< >	**31***
BUN	mg/dL	**22*	22***	**21***	< >	**21***
CREATININE	mg/dL	0.80	0.80	0.81	< >	0.87
CALCIUM	mg/dL	9.1	9.1	9.0	< >	9.0
PHOSPHORUS	mg/dL	**5.1***	**5.3***	< >	--
GLUCOSE	mg/dL	95	95	97	< >	94
ALBUMIN	g/dL	3.8	3.8	3.5	< >	3.6
AST	Units/L	35	--	--	33
ALT	Units/L	25	--	--	21
BILIRUBIN TOTAL	mg/dL	**0.1***	--	--	0.2
ALK PHOS	Units/L	79	--	--	80
< > = values in this interval not displayed.

[table#5]:
Recent Results (from the past 720 hour(s))
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM

	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

**Asses
sment/Plan**

[table#6]:
Assessment and Plan

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6, improving
 # Retropharyngeal and prevertebral fluid collection
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid
- IR will not be able to drain the area.
- PICC placed 1/30. Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).
-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of palms and feet, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1

year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA WNL
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy
Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
**# Bilatera
l leg edema**
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg, Entresto, Farxiga 5mg.
-IV Lasix x 3 doses (2/8-2/10). Resume oral Lasix afterwards.
-Strict I's and O's while on Lasix

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
- Fluid restriction to 2L daily
- Monitor serum sodium.

#Hypertension
-Carvedilol 3.125 mg BID
-Entresto daily

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
- Colchicine dc'ed 1/25/2024 (due to rising creatinine) and resumed 2/9/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Elevated creatinine- resolved
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Resume Lasix 02/01/2024. Held Lasix given increased diuresis and hyponatremia

 # Housing instability
 # Substance use disorder (methamphetamine)

-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage. Medical is currently approved, but still pending SNF placement.

Krishen Wadhwani, DO
PGY-2, Internal Medicine

Cosigned by Martha Strauser, MD at 02/13/2024 2:56 PM PST
Electronically signed by Krishen Wadhwani, DO at 02/13/2024 2:09 PM PST
Electronically signed by Martha Strauser, MD at 02/13/2024 2:56 PM PST

Martha Strauser, MD - 02/13/2024 12:12 PM PST
Formatting of this note might be different from the original.
****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
Known to me 55-year-old homeless female with history of heart failure with reduced ejection fraction of 25 to 35%, alcohol abuse, asthma, methamphetamine use disorder. Admitted on January 12, 2024 with shortness of breath. Found to have a large pleural effusion with atelectasis and empyema. S cervical spine tatus post chest tube placement. Status post intubation due to acute hypoxic hypercarbic respiratory failure. During ICU stay developed A-fib with rapid ventricular response. Patient was extubated her chest tube was removed on January 21, 2024 after treatment with tPA DNase. Later on developed neck pain. Had an MRI of cervical spine which reveals cervical discitis/osteomyelitis from C4-C6 with retro
pharyngeal and prevertebral fluid collection. Evaluated by neurosurgery not recommending any procedure at the present time. Consulted by ID. Is to continue ceftriaxone for at least 8 weeks in conjunction with metronidazole for 2 weeks. Has been evaluated by ENT due to prevertebral fluid collection and they recommended follow-up with CAT scans. Was evaluated by IR who recommended no drainage. Status post PICC line on January 30. Has been on Rocephin since January 25. Is to complete antibiotics on March 22.
Neck pain improved on gabapentin. Was complaining of leg swelling. Restarted on Lasix. Doing better.

Objective
[table#2]:
**OBJECTIVE **
Temp: 36.7 °C (98.1 °F)
Heart Rate: 123
Resp: 18
BP: 145/98
SpO2: 93 %
Temp (24hrs), Avg:36.8 °C (98.2 °F), Min:36.7 °C (98.1 °F), Max:36.9 °C (98.4 °F)

Intake/Output Summary (Last 24 hours) at 2/13/2024 1213
Last data filed at 2/13/2024 1030
[table#3]:
Gross per 24 hour
Intake	690 ml
Output	800 ml
Net	**-110 ml**

SpO2: 93 %
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND
Neuro - normal

MSK - MAE
Skin - no rash, no CCE

Labs
[table#4]:
Results from last 7 days
Lab	Units	02/13/24
0736	02/12/24
0528	02/11/24
2009	02/10/24
0552	02/09/24
1058	02/07/24
0535
SODIUM	mmol/L	139	139	138	138	**132***	< >	**131***
POTASSIUM	mmol/L	4.6	4.6	4.5	4.3	4.5	< >	4.4
CHLORIDE	mmol/L	103	103	102	103	101	< >	100
CO2	mmol/L	**31*	31***	**31***	**31***	**31***	< >	29
BUN	mg/dL	**22*	22***	**21***	**23***	**21***	< >	**20***
CREATININE	mg/dL	0.80	0.80	0.81	0.84	0.87	< >	0.87
CALCIUM	mg/dL	9.1	9.1	9.0	8.8	9.0	< >	8.8
BILIRUBIN TOTAL	mg/dL	**0.1***	--	--	0.2	--	0.2
ALK PHOS	Units/L	79	--	--	80	--	90
ALT	Units/L	25	--	--	21	--	21
AST	Units/L	35	--	--	33	--	32
GLUCOSE	mg/dL	95	95	97	113	94	< >	94
< > = values in this interval not displayed.

[table#5]:
Results from last 7 days
Lab	Units	02/13/24
0736	02/10/24
0552	02/07/24
0535
WBC	K/mcL	5.3	6.0	5.3
HGB	g/dL	**9.8***	**10.1***	**9.2***
HEMATOCRIT	%	**32.2***	**32.4***	**29.5***
PLT	K/mcL	333	377	**446***
NEUTROS PCT AUTO	%	44.1	45.9	44.2
LYMPHS PCT AUTO	%	40.9	41.3	41.7
MONOS PCT AUTO	%	8.9	8.2	9.2
EOS PCT AUTO	%	4.9	3.5	3.2

[table#6]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/28/2024 07:25 AM	130 (H)	70 - 125 mg/dL	Final
01/26/2024 12:06 PM	131 (H)	70 - 125 mg/dL	Final
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final

Imaging Studies
No results found.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
colchicine, 0.6 mg, oral, Daily
dapagliflozin propanediol, 5 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime

furosemide, 40 mg, oral, Daily
gabapentin, 200 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 2 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current Infusion Medications:
D5 %-0.45 % sodium chl
oride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.25 mg, Q4H PRN, 0.25 mg at 02/13/24 0224
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/12/24 1610
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/12/24 2019
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/08/24 0339
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE-acetaminophen, 1 tablet, Q4H PRN, 1 tablet at 02/13/24 0905
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/12/24 2205

Assessment/Plan
[table#7]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with history of alcohol and methamphetamine use disorder, dilated cardiomyopathy with ejection fraction of 25 to 35%. Admitted with shortness of breath and hypoxic hypercarbic respiratory failure requiring intubation. Found to have a right pleural effusion s/p chest tube treatment with tPA DNase. Later on developed neck pain found to have C4-C5 C5-C6 discitis osteomyelitis with prevertebral fluid collection which has been evaluated by ENT and IR. Per ID continue Rocephin to complete 8 weeks as well as metronidazole for 14 days.

-Cervical discitis with osteomyelitis affecting C4-C5 C5-C6-continue Rocephin (1/25/-3/22)to complete 8 weeks of treatment. Continue ENT and ID recommendations.
-Status post acute hypoxic hypercarbic respiratory failure resolved. Status post intubation and extubation on January 14.
-Sepsis secondary to pneumococcus with right-sided empyema. The status post chest tube placement from January 13 to January 21.
-Dilated cardiomyopathy with reduced ejection fraction 25 to 35%-continue carvedilol, Entresto, Lasix, good candidate for SGLT2 inhibitor.
-COPD-stable
-A-fib with rapid ventricular test follows with CHA2DS2-VASc 2 score of 2-patient decided for anticoagulation with Eliquis
-Weakness-PT OT
-Methamphetamine and tobacco abuse-resources in the community have been provided to patient
-Right ankle gout-colchicine
-Oral ulcers a status post Harrop's outbreak-pain much improved with acyclovir
-DVT prophylaxis Eliquis.
-Disposition-SNF
LOS: 32 days
**Disposition Plan: **Quarantined environment pending medical progression.

Code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 02/13/2024 12:15 PM PST

Howie Que, DPM - 02/12/2024 3:50 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of bre
ath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/12/24 0818
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/12/24 0818
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/12/24 0818 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/12/24 0817
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/12/24 1546
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 0.6 mg at 02/12/24 0818
• dapagliflozin propanediol (FARXIGA) tablet 5 mg, 5 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 5 mg at 02/12/24 0818
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/12/24 1546
• furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 40 mg at 02/12/24 0818
• gabapentin (NEURONTIN) capsule 200 mg, 200 mg, oral, TID, Wadhwani, Krishen, DO, 200 mg at 02/12/24 1546
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.25 mg, 0.25 mg, intraVENOUS, Q4H PRN, de Four, Amanda R, MD, 0.25 mg at 02/12/24 0322
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/06/24 0239
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/11/24 1852
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/10/24 1747
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch, 2 patch, transdermal, Q24H, Wadhwani, Krishen, DO, 2 patch at 02/11/24 1744
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/08/24 0339

• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/06/24 1459 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• multivit (Thera M Plus), iron 9
mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/12/24 0818
• oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, de Four, Amanda R, MD, 1 tablet at 02/12/24 0817
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/12/24 0818
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/11/24 2135

Allergies
Lactose

Objective

Vital Signs
[table#1]:
Vitals:
	02/12/24 1112
BP:	118/80
Pulse:	103
Resp:	18
Temp:	36.9 °C (98.4 °F)
SpO2:	99%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine and abx therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/12/24 0528
SODIUM	mmol/L	138
POTASSIUM	mmol/L	4.5
CHLORIDE	mmol/L	102
CO2	mmol/L	**31***
BUN	mg/dL	**21***
CREATININE	mg/dL	0.81
GLUCOSE	mg/dL	97
CALCIUM	mg/dL	9.0

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/12/243:50 PM

Electronically signed by Howie Que, DPM at 02/12/2024 3:51 PM PST

Krishen Wadhwani, DO - 02/12/2024 1:42 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also not
ed to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amount of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/9/2024: Patient is doing well except her stable neck pain no neurologic deficits. She has some swelling in her right lower extremity slightly larger than her left, improved from yesterday.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 118/80 (BP Location: Left arm, Patient Position: Sitting) | Pulse 103 | Temp 36.9 °C (98.4 °F) (Oral) | Resp 18 | Ht 1.676 m (5' 6") | Wt 64.7 kg (142 lb 10.2 oz) | LMP (LMP Unknown) | SpO2 99% | Breastfeeding No | BMI 23.02 kg/m^2
I/O: In: 280 [P.O.:270; I.V.:10]
Out: -
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender

Ext-+1-2 pitting edema around ankles and bilateral feet, R > L
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular- difficult to elicit pulses in bilateral lower extremities.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/12/24
0528	02/11/24
2009	02/10/24
0552	02/09/24
1058	02/07/24
0535
WBC	K/mcL	--	--	6.0	--	5.3
HGB	g/dL	--	--	**10.1***	--	**9.2***
MCV	fL	--	--	100	--	100
PLT	K/mcL	--	--	377	--	**446***
SODIUM	mmol/L	138	138	**132***	< >	**131***
POTASSIUM	mmol/L	4.5	4.3	4.5	< >	4.4
CHLORIDE	mmol/L	102	103	101	< >	100
CO2	mmol/L	**31***	**31***	**31***	< >	29
BUN	mg/dL	**21***	**23***	**21***	< >	**20***
CREATININE	mg/dL	0.81	0.84	0.87	< >	0.87
CALCIUM	mg/dL	9.0	8.8	9.0	< >	8.8
PHOSPHORUS	mg/dL	**5.3***	4.3	-\
-	--	--
GLUCOSE	mg/dL	97	113	94	< >	94
ALBUMIN	g/dL	3.5	3.6	3.6	--	**3.3***
AST	Units/L	--	--	33	--	32
ALT	Units/L	--	--	21	--	21
BILIRUBIN TOTAL	mg/dL	--	--	0.2	--	0.2
ALK PHOS	Units/L	--	--	80	--	90
< > = values in this interval not displayed.

[table#5]:
Recent Results (from the past 720 hour(s))
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM

	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

X-ray hip right 2-3 views

Result Date: 2/12/2024
IMPRESSION: 1. Mild degenerative changes at right hip joint. No acute fracture detected. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 2/11/2024
IMPRESSION: No deep venous thrombosis detected in either leg. release

Assessment/Plan

[table#6]:
Assessment and Plan

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6, improving
 # Retropharyngeal and prevertebral fluid collection
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid
- IR will not be able to drain the area.
- PICC placed 1/30. Rocephin for 56 days (1/25-3/21) and Flagyl for 14
days: (1/25-2/8).

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).
-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of palms and feet, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA WNL
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy
Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
 # Bilateral leg edema
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg, Entresto. Start Farxiga 5mg.
-IV Lasix x 3 doses (2/8-2/10). Resume oral Lasix afterwards.
-Strict I's and O's while on Lasix

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
- Fluid restriction to 2L daily
- Monitor serum sodium.

#Hypertension
-Carvedilol 3.125 mg BID
-Entresto daily

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
- Colchicine dc'ed 1/25/2024 (due to rising creatinine
) and resumed 2/9/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Elevated creatinine- resolved
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Resume Lasix 02/01/2024. Held Lasix given increased diuresis and hyponatremia

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

Krishen Wadhwani, DO

Cosigned by Amanda de Four, MD at 02/12/2024 10:01 PM PST
Electronically signed by Krishen Wadhwani, DO at 02/12/2024 1:50 PM PST
Electronically signed by Amanda de Four, MD at 02/12/2024 10:01 PM PST

Associated attestation - Amanda de Four, MD - 02/12/2024 10:01 PM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident on rounds. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note below as needed. I agree with and verify the findings and plan outlined except as noted below

Elizabeth Tina Petruccio is a 55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amount of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

Krishen Wadhwani, DO - 02/11/2024 5:08 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

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She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/9/2024:
Patient is doing well except her stable neck pain no neurologic deficits. She has some swelling in her right lower extremity slightly larger than her left, improved from yesterday.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 131/77 (BP Location: Left arm, Patient Position: Sitting) | Pulse 98 | Temp 37.1 °C (98.8 °F) (Oral) | Resp 16 | Ht 1.676 m (5' 6") | Wt 64.7 kg (142 lb 10.2 oz) | LMP (LMP Unknown) | SpO2 98% | Breastfeeding No | BMI 23.02 kg/m²
I/O: In: 990 [P.O.:990]
Out: 1350 [Urine:1350]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+2 pitting edema around ankles and bilateral feet
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular- difficult to elicit pulses in bilateral lower extremities.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/10/24
0552	02/09/24
1058	02/07/24
0535
WBC	K/mcL	6.0	--	5.3
HGB	g/dL	**10.1***	--	**9.2***
MCV	fL	100	--	100
PLT	K/mcL	377	--	**446***
SODIUM	mmol/L	**132***	**136***	**131***
POTASSIUM	mmol/L	4.5	4.5	4.4
CHLORIDE	mmol/L	101	100	100
CO2	mmol/L	**31***	**31***	29
BUN	mg/dL	**21***	**22***	**20***
CREATININE	mg/dL	0.87	1.00	0.87
CALCIUM	mg/dL	9.0	8.9	8.8
GLUCOSE	mg/dL	94	99	94
ALBUMIN	g/dL	3.6	--	**3.3***
AST	Units/L	33	--	32
ALT	Units/L	21	--	21
BILIRUBIN TOTAL	mg/dL	0.2	--	0.2
ALK PHOS	Units/L	80	--	90

[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected

| | Influenza B PCR | Not Detected |
| **COVID-19, Rapid (PCR/NAAT COMBO)** |
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Coll
ection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	

| | | NO Methicillin/Oxacillin Resistant Staphylococcus aureus |
| **Wound/Fluid/Tissue Culture with stain** |
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

X-ray hip right 2-3 views

Result Date: 2/11/2024
IMPRESSION: 1. Mild degenerative changes at right hip joint. No acute fracture detected. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 2/11/2024
IMPRESSION: No deep venous thrombosis detected in either leg. release

Assessment/Plan

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Assessment and Plan

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 # Cervical discitis/osteomyelitis C4-C6, improving
 # Retropharyngeal and prevertebral fluid collection
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid
- IR will not be able to drain the area.
- PICC placed 1/30. Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomyci
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-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).

-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of palms and feet, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA WNL
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy
Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
 # Bilateral leg edema
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg, Entresto. Start Farxiga 5mg.
-IV Lasix x 3 doses (2/8-2/10). Resume oral Lasix afterwards.
-Strict I's and O's while on Lasix

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1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
- Fluid restriction to 2L daily
- Monitor serum sodium.

#Hypertension
-Carvedilol 3.125 mg BID
-Entresto daily

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
- Colchicine dc'ed 1/25/2024 (due to rising creatinine) and resumed 2/9/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely th
at patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Elevated creatinine- resolved
 # Possible contrast-induced AKI -resolved.

-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Resume Lasix 02/01/2024. Held Lasix given increased diuresis and hyponatremia

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

Krishen Wadhwani, DO

Cosigned by Amanda de Four, MD at 02/11/2024 7:15 PM PST
Electronically signed by Krishen Wadhwani, DO at 02/11/2024 5:11 PM PST
Electronically signed by Amanda de Four, MD at 02/11/2024 7:15 PM PST

Associated attestation - Amanda de Four, MD - 02/11/2024 7:15 PM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident on rounds. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note below as needed. I agree with and verify the findings and plan outlined except as noted below

Howie Que, DPM - 02/11/2024 11:45 AM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/11/24 0813
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/11/24 0812
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/11/24 0812 **AND** bisaCODYL

(DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samue
l, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/11/24 0812
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/10/24 1642
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 0.6 mg at 02/11/24 0812
• dapagliflozin propanediol (FARXIGA) tablet 5 mg, 5 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 5 mg at 02/11/24 0812
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/11/24 0756
• furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 40 mg at 02/11/24 0812
• gabapentin (NEURONTIN) capsule 200 mg, 200 mg, oral, TID, Wadhwani, Krishen, DO
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.25 mg, 0.25 mg, intraVENOUS, Q4H PRN, de Four, Amanda R, MD, 0.25 mg at 02/11/24 0509
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/06/24 0239
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/11/24 0202
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/10/24 1747
• [START ON 2/12/2024] lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch, 2 patch, transdermal, Q24H, Wadhwani, Krishen, DO
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/08/24 0339
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/06/24 1459 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/11/24 0812
• oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, de Four, Amanda R, MD, 1 tablet at 02/11/24 0223
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/11/24 0812
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/10/24 2228

Allergies
Lactose

Objective

Vital Signs
[table#1]:
Vitals:
	02/11/24 1133
BP:	128/70
Pulse:	108
Resp:	16
Temp:	36.9 °C (98.4 °F)
SpO2:	98%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pat
hology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine and abx therapy.

[table#2]:
Results from last 7 days
Lab	Units	02/10/24 0552
SODIUM	mmol/L	**132***
POTASSIUM	mmol/L	4.5
CHLORIDE	mmol/L	101
CO2	mmol/L	**31***
BUN	mg/dL	**21***
CREATININE	mg/dL	0.87
GLUCOSE	mg/dL	94
CALCIUM	mg/dL	9.0

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/11/2411:46 AM

Electronically signed by Howie Que, DPM at 02/11/2024 11:46 AM PST

Your Scripps Health Team - 02/10/2024 4:25 PM PST
Formatting of this note might be different from the original.
Admitted for SOB & Sepsis 2/2 empyema. S/p chest tube placed 1/13 & removed 1/21. IV/PO abx. S/p intubation 1/13 & extubation 1/14. Hx of homelessness, etoh, & meth use. Walked 200'. w/ FWW per 2/9 PT notes Stays w/ son Anthony in Huntington Beach shelters or travels in MX, indp pta. Dtr Erika lives in NJ. Medi-cal only--HMO application sent 1/22. Not candidate for alt bed. Dcp: SNF for IV abx for total 56 days. MCAL call list. PASRR L1 done No accepting snf. Continue to search. Updated clinicals on ecin and sent referrals.--Cleofe B. Inpatient CM/RN/BSN Scripps Mercy Hospital Chula Vista

Electronically signed by Your Scripps Health Team at 02/10/2024 4:27 PM PST

Amanda de Four, MD - 02/10/2024 12:06 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amount of partly
enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/9/2024: Patient is doing well except her stable neck pain no neurologic deficits. She has some swelling in her right lower extremity slightly larger than her left

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 107/75 (BP Location: Right leg, Patient Position: Sitting) | Pulse 114 | Temp 37.5 °C (99.5 °F) (Oral) | Resp 18 | Ht 1.676 m (5' 6") | Wt 64.4 kg (141 lb 15.6 oz) | LMP (LMP Unknown) | SpO2 97% | Breastfeeding No | BMI 22.92 kg/m²
I/O: In: 900 [P.O.:900]
Out: 1525 [Urine:1525]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+2 pitting edema around ankles and bilateral feet
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular- difficult to elicit pulses in bilateral lower extremities.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/10/24
0552	02/09/24
1058	02/07/24
0535
WBC	K/mcL	6.0	--	5.3
HGB	g/dL	**10.1***	--	**9.2***
MCV	fL	100	--	100
PLT	K/mcL	377	--	**446***
SODIUM	mmol/L	**132***	**136***	**131***
POTASSIUM	mmol/L	4.5	4.5	4.4
CHLORIDE	mmol/L	101	100	100
CO2	mmol/L	**31***	**31***	29
BUN	mg/dL	**21***	**22***	**20***
CREATININE	mg/dL	0.87	1.00	0.87
CALCIUM	mg/dL	9.0	8.9	8.8
GLUCOSE	mg/dL	94	99	94
ALBUMIN	g/dL	3.6	--	**3.3***
AST	Units/L	33	--	32
ALT	Units/L	21	--	21
BILIRUBIN TOTAL	mg/dL	0.2	--	0.2
ALK PHOS	Units/L	80	--	90

[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph
Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM

	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#6]:
Assessment and Plan

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # Retropharyngeal and prevertebral fluid collection
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid
- IR will not be able to drain the area.
- PICC placed 1/30. Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).

**# Acute
hypoxic respiratory failure - resolved**
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs

-Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).
-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of palms and feet, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA WNL
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy
Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
 # Bilateral leg edema
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg, Entresto. Start Farxiga 5mg.
-IV Lasix x 3 doses (2/8-2/10). Resume oral Lasix afterwards.
-Strict I's and O's while on Lasix

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
- Fluid restriction to 2L daily
- Monitor serum sodium.

#Hypertension
-Carvedilol 3.125 mg BID
-Entresto daily

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
- Colchicine dc'ed 1/25/2024 (due to rising creatinine) and resumed 2/9/2024.

**# Or
al Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Elevated creatinine- resolved

 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Resume Lasix 02/01/2024. Held Lasix given increased diuresis and hyponatremia

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

Amanda R De Four, MD

Electronically signed by Amanda de Four, MD at 02/10/2024 6:18 PM PST

Howie Que, DPM - 02/10/2024 11:44 AM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/11/24 0813
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/11/24 0812
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/11/24 0812 **A
ND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/11/24 0812
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/10/24 1642
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 0.6 mg at 02/11/24 0812
• dapagliflozin propanediol (FARXIGA) tablet 5 mg, 5 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 5 mg at 02/11/24 0812
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime,

Ani, Samuel, MD, 15 mL at 02/11/24 0756
• furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 40 mg at 02/11/24 0812
• gabapentin (NEURONTIN) capsule 200 mg, 200 mg, oral, TID, Wadhwani, Krishen, DO
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.25 mg, 0.25 mg, intraVENOUS, Q4H PRN, de Four, Amanda R, MD, 0.25 mg at 02/11/24 0509
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/06/24 0239
• ipratropium-albuterol (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/11/24 0202
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/10/24 1747
• [START ON 2/12/2024] lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch, 2 patch, transdermal, Q24H, Wadhwani, Krishen, DO
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/08/24 0339
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/06/24 1459 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/11/24 0812
• oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, de Four, Amanda R, MD, 1 tablet at 02/11/24 0223
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/11/24 0812
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/10/24 2228

Allergies
Lactose

Objective

Vital Signs
[table#1]:
Vitals:
	02/11/24 1133
BP:	128/70
Pulse:	108
Resp:	16
Temp:	36.9 °C (98.4 °F)
SpO2:	98%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surround
ing erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine and

abx therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/10/24
0552
SODIUM	mmol/L	**132***
POTASSIUM	mmol/L	4.5
CHLORIDE	mmol/L	101
CO2	mmol/L	**31***
BUN	mg/dL	**21***
CREATININE	mg/dL	0.87
GLUCOSE	mg/dL	94
CALCIUM	mg/dL	9.0

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/11/2411:45 AM

Electronically signed by Howie Que, DPM at 02/11/2024 11:45 AM PST

Howie Que, DPM - 02/09/2024 2:47 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/09/24 0922
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/09/24 0858
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, S
amuel, MD, 100 mg at 01/26/24 1853

• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/09/24 0858 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/09/24 0918
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/08/24 1553
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 0.6 mg at 02/09/24 1339
• dapagliflozin propanediol (FARXIGA) tablet 5 mg, 5 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 5 mg at 02/09/24 1127
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/09/24 1242
• furosemide (LASIX) injection 40 mg, 40 mg, intraVENOUS, Daily, Umukoro, Emuejevuoke Anastasia, DO, 40 mg at 02/09/24 0857
• [START ON 2/11/2024] furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO
• gabapentin (NEURONTIN) capsule 100 mg, 100 mg, oral, TID, Umukoro, Emuejevuoke Anastasia, DO, 100 mg at 02/09/24 1426
• HYDROcodone-acetaminophen (NORCO) 10-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 tablet at 02/09/24 1432
• HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, Umukoro, Emuejevuoke Anastasia, DO
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/06/24 0239
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/08/24 2318
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/09/24 0434
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/08/24 1553
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/08/24 0339
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/06/24 1459 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/09/24 0858
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/09/24 0858
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/08/24 2310

Allergies
Lactose

Objective

Vital Signs
[table#1]:
Vitals:
	02/09/24 1100
BP:	112/80
Pulse:	
Resp:	18
Temp:	36.9 °C (98.4 °F)
SpO2:	

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect
of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout

2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine and abx therapy.

[table#2]:
Results from last 7 days
Lab	Units	02/09/24 1058
SODIUM	mmol/L	**136***
POTASSIUM	mmol/L	4.5
CHLORIDE	mmol/L	100
CO2	mmol/L	**31***
BUN	mg/dL	**22***
CREATININE	mg/dL	1.00
GLUCOSE	mg/dL	99
CALCIUM	mg/dL	8.9

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/09/242:47 PM

Electronically signed by Howie Que, DPM at 02/09/2024 2:48 PM PST

Emuejevuoke Umukoro, DO - 02/09/2024 12:00 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had M
RI on 1/26 with contrast that was very limited but showed a large amount of partly enhancing posterior nasopharyngeal,

prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/9/2024: No acute events overnight. She reports improvement in lower extremity swelling and stable right shoulder and neck pain. Denies any other complaints. 2.9 L urine output over the last 24 hours. Continue Lasix, start colchicine and Farxiga.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 112/80 (BP Location: Left arm, Patient Position: Sitting) | Pulse 91 | Temp 36.9 °C (98.4 °F) (Oral) | Resp 18 | Ht 1.676 m (5' 6") | Wt 64.5 kg (142 lb 3.2 oz) | LMP (LMP Unknown) | SpO2 94% | Breastfeeding No | BMI 22.95 kg/m^2
I/O: In: 1070 [P.O.:1050; I.V.:20]
Out: 2900 [Urine:2900]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+2 pitting edema around ankles and bilateral feet
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular- difficult to elicit pulses in bilateral lower extremities.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/09/24
1058	02/07/24
0535	02/06/24
0529
WBC	K/mcL	--	5.3	5.0
HGB	g/dL	--	**9.2***	**9.1***
MCV	fL	--	100	100
PLT	K/mcL	--	**446***	**472***
SODIUM	mmol/L	**136***	**131***	**132***
POTASSIUM	mmol/L	4.5	4.4	4.1
CHLORIDE	mmol/L	100	100	103
CO2	mmol/L	**31***	29	25
BUN	mg/dL	**22***	**20***	16
CREATININE	mg/dL	1.00	0.87	0.71
CALCIUM	mg/dL	8.9	8.8	8.8
GLUCOSE	mg/dL	99	94	103
ALBUMIN	g/dL	--	**3.3***	**3.3***
AST	Units/L	--	32	36
ALT	Units/L	--	21	26
BILIRUBIN TOTAL	mg/dL	--	0.2	0.2
ALK PHOS	Units/L	--	90	84

[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected

| | Influenza B PCR | Not Detected |
| **COVID-19, Rapid (PCR/NAAT COMBO)** |
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Va
lue
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	

| | | NO Methicillin/Oxacillin Resistant Staphylococcus aureus |
| **Wound/Fluid/Tissue Culture with stain** |
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#6]:
Assessment and Plan

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # Retropharyngeal and prevertebral fluid collection
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persiste
nt prevertebral fluid
- IR will not be able to drain the area.
- PICC placed 1/30. Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-3/21) and Flagyl for 14 days: (1/25-2/8).
-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of palms and feet, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy
Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
 # Bilateral leg edema
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg, Entresto. Start Farxiga 5mg.
-IV Lasix x 3 doses (2/8-2/10). Resume oral Lasix afterwards.
-Strict I's and O's while on Lasix

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
- Fluid restriction to 2L daily
- Monitor serum sodium.

#Hypertension
-Carvedilol 3.125 mg BID
-Entresto daily

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yield
ed insufficient aspirate for culture. Recommended continuation of colchicine treatment .
- Colchicine dc'ed 1/25/2024 (due to rising creatinine) and resumed 2/9/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Elevated creatinine- resolved
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Resume Lasix 02/01/2024. Held Lasix given increased diuresis and hyponatremia

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

Resident: Emuejevuoke Anastasia Umukoro, DO

Attending: Martha Dalia Strauser, MD

Cosigned by Martha Strauser, MD at 02/13/2024 2:56 PM PST
Electronically signed by Emuejevuoke Umukoro, DO at 02/09/2024 12:13 PM PST
Electronically signed by Emuejevuoke Umukoro, DO at 02/09/2024 1:26 PM PST
Electronically signed by Martha Strauser, MD at 02/13/2024 2:56 PM PST

Martha Strauser, MD - 02/09/2024 9:08 AM PST
Formatting of this note might be different from the original.
****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
Known to me 55-year-old homeless female with history of heart failure with reduced ejection fraction of 25 to 35%, alcohol abuse, asthma, methamphetamine use disorder. Admitted on January 12, 2024 with shortness of breath. Found to have a large pleural effusion with atelectasis and empyema. S cervical spine tatus post chest tube placement. Status post intubation due to acute hypoxic hypercarbic respiratory failure. During ICU stay developed A-fib with rapid ventricular response. Patient was extubated her chest tube was removed on January 21, 2024 after treatment with tPA DNase. Later on developed neck pain. Had an MRI of cervical spine which reveals cervical discitis/osteomyelitis from C4-C6 with retropharyngeal and prevertebral fluid collection. Evaluated by neurosurgery not recommending any procedure at the present time. Consulted by ID. Is to continue ceftriaxone for at least 8 weeks in conjunction with metronidazole for 2 weeks. Has been evaluated by ENT due to prevertebral fluid collection and they recommended follow-up with CAT scans. Was evaluated by IR who recommended no drainage. Status post PICC line on January 30. Has been on Rocephin since January 25.
Neck pain improved on gabapentin. Was complaining of leg swelling. Restarted on Lasix.

Objective
[table#2]:
**OBJECTIVE **
Temp: 36.9 °C (98.4 °F)
Heart Rate: 91
Resp: 18
BP: 110/83
SpO2: 93 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 0 L/min
Temp (24hrs), Avg:36.7 °C (98.1 °F), Min:36.4 °C (97.5 °F), Max:36.9 °C (98.4 °F)

Intake/Output Summary (Last 24 hours) at 2/9/2024 0908
Last data filed at 2/9/2024 0833
[table#3]:
Gross per 24 hour
Intake	950 ml
Output	2900 ml
Net	**-1950 ml**

SpO2: 93 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 0 L/min
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND

Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs
[table#4]:
Results from last 7 days
Lab	Units	02/07/24
0535	02/06/24
0529	02/05/24
0553
SODIUM	mmol/L	**131***	**132***
131*
POTASSIUM	mmol/L	4.4	4.1	4.5
CHLORIDE	mmol/L	100	103	98
CO2	mmol/L	29	25	29
BUN	mg/dL	**20***	16	15
CREATININE	mg/dL	0.87	0.71	0.75
CALCIUM	mg/dL	8.8	8.8	8.7
BILIRUBIN TOTAL	mg/dL	0.2	0.2	**0.1***
ALK PHOS	Units/L	90	84	107
ALT	Units/L	21	26	22
AST	Units/L	32	36	**39***
GLUCOSE	mg/dL	94	103	94

[table#5]:
Results from last 7 days
Lab	Units	02/07/24
0535	02/06/24
0529	02/05/24
0553
WBC	K/mcL	5.3	5.0	5.3
HGB	g/dL	**9.2***	**9.1***	**8.8***
HEMATOCRIT	%	**29.5***	**30.5***	**29.0***
PLT	K/mcL	**446***	**472***	**515***
NEUTROS PCT AUTO	%	44.2	42.6	49.7
LYMPHS PCT AUTO	%	41.7	42.5	35.5
MONOS PCT AUTO	%	9.2	10.5	10.2
EOS PCT AUTO	%	3.2	2.8	3.0

[table#6]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/28/2024 07:25 AM	130 (H)	70 - 125 mg/dL	Final
01/26/2024 12:06 PM	131 (H)	70 - 125 mg/dL	Final
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final

Imaging Studies
No results found.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 40 mg, intraVENOUS, Daily
[Held by provider] furosemide, 40 mg, oral, Daily
gabapentin, 100 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H

lidocaine-menthol, 1 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current Infusion Medications:
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROcodone-acetaminophen, 1 tablet, Q4H PRN, 1 tablet at 02/09/24 0434
HYDROcodone-acetaminophen, 1 tablet, Q4H PRN
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/06/24 0239
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/08/24 2318
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/08/24 0339
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/08/24 2310

Assessment/Plan
[table#7]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with history of alcohol and methamphetamine use disorder, dilated cardiomyopathy with ejection fraction of 25 to 35%. Admitted with shortness of breath and hypoxic hypercarbic respiratory failure requiring intubation. Found to have a right pleural effusion s/p chest tube treatment with tPA DNase. Later on developed neck pain found to have C4-C5 C5-C6 discitis osteomyelitis with prevertebral fluid collection which has been evaluated by ENT and IR. Per ID continue Rocephin to complete 8 weeks as well as metronidazole for 14 days.

-Cervical discitis with osteomyelitis affecting C4-C5 C5-C6-continue Rocephin (1/26/-3/22)to complete 8 weeks of treatment. Continue ENT and ID recommendations.
-Status post acute hypoxic hypercarbic respiratory failure resolved. Status post intubatio
n and extubation on January 14.
-Sepsis secondary to pneumococcus with right-sided empyema. The status post chest tube placement from January 13 to January 21.
-Dilated cardiomyopathy with reduced ejection fraction 25 to 35%-continue carvedilol, Entresto, Lasix, good candidate for SGLT2 inhibitor.
-COPD-stable
-A-fib with rapid ventricular test follows with CHA2DS2-VASc 2 score of 2-patient decided for anticoagulation with Eliquis
-Weakness-PT OT
-Methamphetamine and tobacco abuse-resources in the community have been provided to patient
-Right ankle gout-colchicine
-Oral ulcers a status post Harrop's outbreak-pain much improved with acyclovir
-DVT prophylaxis Eliquis.
-Disposition-SNF
LOS: 28 days
**Disposition Plan: **Quarantined environment pending medical progression.

Code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 02/09/2024 11:49 AM PST

Emuejevuoke Umukoro, DO - 02/08/2024 2:04 PM PST

Formatting of this note might be different from the original.

[image]

Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amount of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/8/2024: Patient is lying comfortably in bed. No acute events overnight. Intermittent right shoulder and neck pain relieved with medications and rest/heat packs. Denies any other complaints. 1.4 L urine output over the last 24 hours.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 108/76 (BP Location: Left arm, Patient Position: Lying) | Pulse 107 | Temp 36.4 °C (97.5 °F) (Oral) | Resp 19 | Ht 1.676 m (5' 6") | Wt 65.3 k
g (143 lb 14.4 oz) | LMP (LMP Unknown) | SpO2 100% | Breastfeeding No | BMI 23.23 kg/m²
I/O: In: 760 [P.O.:720; I.V.:40]
Out: 1390 [Urine:1390]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+2 pitting edema around ankles and bilateral feet
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular- difficult to elicit pulses in bilateral lower extremities..
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:

Results from last 7 days
Lab	Units	02/07/24
0535	02/06/24
0529
WBC	K/mcL	5.3	5.0
HGB	g/dL	**9.2***	**9.1***
MCV	fL	100	100
PLT	K/mcL	**446***	**472***
SODIUM	mmol/L	**131***	**132***
POTASSIUM	mmol/L	4.4	4.1
CHLORIDE	mmol/L	100	103
CO2	mmol/L	29	25
BUN	mg/dL	**20***	16
CREATININE	mg/dL	0.87	0.71
CALCIUM	mg/dL	8.8	8.8
GLUCOSE	mg/dL	94	103
ALBUMIN	g/dL	**3.3***	**3.3***
AST	Units/L	32	36
ALT	Units/L	21	26
BILIRUBIN TOTAL	mg/dL	0.2	0.2
ALK PHOS	Units/L	90	84

[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final

| **Wound/Fluid/Tissue Culture with stain** |
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
**Urine cul
ture**
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

X-ray ankle right 3 or more views

Result Date: 2/7/2024
IMPRESSION: 1. Marked soft tissue swelling at the ankle and hindfoot and midfoot and forefoot 2. Small plantar calcaneal spur 3. No fractures or dislocations identified release

X-ray ankle left 3 or more views

Result Date: 2/7/2024
IMPRESSION: 1. Severe soft tissue swelling around the ankle midfoot and forefoot correlate clinically for cellulitis. No radiographic evidence of osteomyelitis. Midfoot degenerative changes 2. No soft tissue emphysema release

Assessment/Plan

[table#6]:
Assessment and Plan

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024.

CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # Retropharyngeal and prevertebral fluid collection
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid
- IR will not be able to drain the area.
-PICC placed 1/30. Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-2/8).

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is cli nically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-2/8).
-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of palms and feet, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy
Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
 # Bilateral leg edema
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg, Entresto
-IV Lasix x 3 doses (2/8-2/10). Resume oral Lasix afterwards.
-Strict I's and O's while on Lasix

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
- Fluid restriction to 2L daily
- Monitor serum sodium.

#Hypertension
-Carvedilol 3.125 mg p.o. twice daily and Entresto

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Elevated creatinine- resolved
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Resume Lasix 02/01/2024. Held Lasix given increased diuresis and hyponatremia

 # Housing instability
**#
Substance use disorder (methamphetamine)**
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

Resident: Emuejevuoke Anastasia Umukoro, DO

Attending: Martha Dalia Strauser, MD

Cosigned by Martha Strauser, MD at 02/09/2024 9:08 AM PST
Electronically signed by Emuejevuoke Umukoro, DO at 02/08/2024 2:10 PM PST
Electronically signed by Emuejevuoke Umukoro, DO at 02/08/2024 2:11 PM PST
Electronically signed by Martha Strauser, MD at 02/09/2024 9:08 AM PST

Howie Que, DPM - 02/08/2024 10:01 AM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was

insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/07/24 2150
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/07/24 2150
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/07/24 2149 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/07/24 2150
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/07/24 1606
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily
PRN, Ani, Samuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/08/24 0642
• furosemide (LASIX) injection 40 mg, 40 mg, intraVENOUS, Daily, Umukoro, Emuejevuoke Anastasia, DO
• [Held by provider] furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 40 mg at 02/07/24 0910
• gabapentin (NEURONTIN) capsule 100 mg, 100 mg, oral, TID, Umukoro, Emuejevuoke Anastasia, DO, 100 mg at 02/07/24 2150
• HYDROcodone-acetaminophen (NORCO) 10-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 tablet at 02/08/24 0743
• HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, Umukoro, Emuejevuoke Anastasia, DO
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/06/24 0239
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/08/24 0310
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/08/24 0340
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/07/24 1606
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/08/24 0339
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/06/24 1459 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Ani, Samuel, MD, 500 mg at 02/07/24 2150
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/07/24 0910
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/07/24 2150
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/07/24 2150

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
| **Vitals:** |

| --- |
	02/08/24 0740
BP:	101/69
Pulse:	106
Resp:	20
Temp:	36.6 °C (97.9 °F)
SpO2:	96%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term seq
uela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine and abx therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/07/24 0535
SODIUM	mmol/L	**131***
POTASSIUM	mmol/L	4.4
CHLORIDE	mmol/L	100
CO2	mmol/L	29
BUN	mg/dL	**20***
CREATININE	mg/dL	0.87
GLUCOSE	mg/dL	94
CALCIUM	mg/dL	8.8

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/08/2410:02 AM

Electronically signed by Howie Que, DPM at 02/08/2024 11:41 AM PST

Formatting of this note might be different from the original.
NUTRITION CARE CHECK NOTE:

Patient average PO intake 94% x 9 meals, Weight currently stable. Current diet is adequate and appropriate:

[table#1]:
|
|
| --- |
| [table#1.1] |

[table#1.1]:
Dietary Orders
(From admission, onward)

[table#2]:
|
|
| --- |

[table#3]:
| | [table#3.1]
[table#3.2] |
| --- | --- |

[table#3.1]:
	Start			Ordered
	02/04/24 0904		[table#3.1.1]	02/04/24 0904

[table#3.1.1]:
Combination Diet Regular; 2000 mL Diet effective now
[table#3.1.1.1]

[table#3.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Fluid Restriction - Oral (mL/day):	2000 mL	

[table#3.2]:

[table#4]:
.

Allergies:
[table#5]:
Allergies
Allergen	Reactions
•	Lactose Intolerant [Lactose]	Other (see comments)
		Patient having diarrhea; only fluid milk - all other lactose items ok

Last BM Date: 02/07/24, Stool Appearance: Unable to assess. Labs reviewed. Continue to monitor and offer nutrition interventions as needed.

**Sarah P, RD **
Clinical Dietitian
RD Office: 312-7176

Electronically signed by Your Scripps Health Team at 02/08/2024 9:19 AM PST
Martha Strauser, MD - 02/08/2024 8:47 AM PST

Formatting of this note might be different from the original.
****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
Known to me 55-year-old homeless female with history of heart failure with reduced ejection fraction of 25 to 35%, alcohol abuse, asthma, methamphetamine use disorder. Admitted on January 12, 2024 with shortness of breath. Found to have a large pleural effusion with atelectasis and empyema. S cervical spine tatus post chest tube placement. Status post intubation due to acute hypoxic hypercarbic respiratory failure. During ICU stay developed A-fib with rapid ventricular response. Patient was extubated her chest tube was removed on January 21, 2024 after treatment with tPA DNase. Later on developed neck pain. Had an MRI of cervical spine which reveals cervical discitis/osteomyelitis from C4-C6 with retropharyngeal and prevertebral fluid collection. Evaluated by neurosurgery not recommending any procedure at the present time. Consulted by ID. Is to continue ceftriaxone for at least 8 weeks in conjunction with metronidazole for 2 weeks. Has been evaluated by ENT due to prevertebral fluid collection and they recommended follow-up with CAT scans. Was evaluated by IR who recommended no drainage. Status post PICC line on January 30. Has been on Rocephin since January 25.
Neck pain improved on gabapentin. Was complaining of leg swelling. Restarted on Lasix.

Objective
[table#2]:
**OBJECTIVE **
T
emp: 36.6 °C (97.9 °F)
Heart Rate: 106
Resp: 20
BP: 101/69
SpO2: 96 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 0 L/min
Temp (24hrs), Avg:36.7 °C (98 °F), Min:36.4 °C (97.5 °F), Max:37.1 °C (98.8 °F)

Intake/Output Summary (Last 24 hours) at 2/8/2024 0848
Last data filed at 2/8/2024 0740
[table#3]:
Gross per 24 hour
Intake	1000 ml
Output	1190 ml
Net	**-190 ml**

SpO2: 96 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 0 L/min
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND
Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs
[table#4]:
Results from last 7 days
Lab	Units	02/07/24
0535	02/06/24
0529	02/05/24
0553
SODIUM	mmol/L	**131***	**132***	**131***
POTASSIUM	mmol/L	4.4	4.1	4.5
CHLORIDE	mmol/L	100	103	98
CO2	mmol/L	29	25	29

BUN	mg/dL	**20***	16	15
CREATININE	mg/dL	0.87	0.71	0.75
CALCIUM	mg/dL	8.8	8.8	8.7
BILIRUBIN TOTAL	mg/dL	0.2	0.2	**0.1***
ALK PHOS	Units/L	90	84	107
ALT	Units/L	21	26	22
AST	Units/L	32	36	**39***
GLUCOSE	mg/dL	94	103	94

[table#5]:
Results from last 7 days
Lab	Units	02/07/24
0535	02/06/24
0529	02/05/24
0553
WBC	K/mcL	5.3	5.0	5.3
HGB	g/dL	**9.2***	**9.1***	**8.8***
HEMATOCRIT	%	**29.5***	**30.5***	**29.0***
PLT	K/mcL	**446***	**472***	**515***
NEUTROS PCT AUTO	%	44.2	42.6	49.7
LYMPHS PCT AUTO	%	41.7	42.5	35.5
MONOS PCT AUTO	%	9.2	10.5	10.2
EOS PCT AUTO	%	3.2	2.8	3.0

[table#6]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/28/2024 07:25 AM	130 (H)	70 - 125 mg/dL	Final
01/26/2024 12:06 PM	131 (H)	70 - 125 mg/dL	Final
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final

Imaging Studies
X-ray ankle right 3 or more views

Result Date: 2/7/2024
IMPRESSION: 1. Marked soft tissue swelling at the ankle and hindfoot and midfoot and forefoot 2. Small plantar calcaneal spur 3. No fractures or dislocations identified release

X-ray ankle left 3 or more views

Result Date: 2/7/2024
IMPRESSION: 1. Severe soft tissue swelling around the ankle midfoot and forefoot correlate clinically for cellulitis. No radiographic evidence of osteomyelitis. Midfoot degenerative changes 2. No soft tissue emphysema release

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 40 mg, intraVENOUS, Daily
[Held by provider] furosemide, 40 mg, oral, Daily
gabapentin, 100 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current Infusion Medications:

dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Da
ily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROcodone-acetaminophen, 1 tablet, Q4H PRN, 1 tablet at 02/08/24 0743
HYDROcodone-acetaminophen, 1 tablet, Q4H PRN
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/06/24 0239
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/08/24 0310
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/08/24 0339
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/07/24 2150

Assessment/Plan
[table#7]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with history of alcohol and methamphetamine use disorder, dilated cardiomyopathy with ejection fraction of 25 to 35%. Admitted with shortness of breath and hypoxic hypercarbic respiratory failure requiring intubation. Found to have a right pleural effusion s/p chest tube treatment with tPA DNase. Later on developed neck pain found to have C4-C5 C5-C6 discitis osteomyelitis with prevertebral fluid collection which has been evaluated by ENT and IR. Per ID continue Rocephin to complete 8 weeks as well as metronidazole for 14 days.

-Cervical discitis with osteomyelitis affecting C4-C5 C5-C6-continue Rocephin to complete 8 weeks of treatment. Continue ENT and ID recommendations.
-Status post acute hypoxic hypercarbic respiratory failure resolved. Status post intubation and extubation on January 14.
-Sepsis secondary to pneumococcus with right-sided empyema. The status post chest tube placement from January 13 to January 21.
-Dilated cardiomyopathy with reduced ejection fraction 25 to 35%-continue carvedilol, Entresto, Lasix, good candidate for SGLT2 inhibitor.
-COPD-stable
-A-fib with rapid ventricular test follows with CHA2DS2-VASc 2 score of 2-patient decided for anticoagulation with Eliquis
-Weakness-PT OT
-Methamphetamine and tobacco abuse-resources in the community have been provided to patient
-Right ankle gout-colchicine
-Oral ulcers a status post Harrop's outbreak-pain much improved with acyclovir
-DVT prophylaxis Eliquis.
-Disposition-SNF
LOS: 27 days
**Disposition Plan: **Quarantined environment pending medical progression.

Code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 02/08/2024 11:28 AM PST

Emuejevuoke Umukoro, DO - 02/07/2024 12:53 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of ri
ght foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amount of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/7/2024: Patient is sitting comfortably in her chair. She reports improvement in her shoulder pain and stable neck pain. She admits mild improvement in bilateral foot swelling. Denies any other complaints.
She had 1.7 L urine output after 1 dose of IV Lasix 40 mg yesterday. Change labs to every 3 days.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 113/74 (BP Location: Left arm, Patient Position: Lying) | Pulse 89 | Temp 36.4 °C (97.5 °F) (Oral) | Resp 19 | Ht 1.676 m (5' 6") | Wt 67 kg (147 lb 11.3 oz) | LMP (LMP Unknown) | SpO2 100% | Breastfeeding No | BMI 23.84 kg/m²
I/O: In: 618 [P.O.:598; I.V.:20]
Out: 1690 [Urine:1690]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic. Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+2 pitting edema around ankles and bilateral feet
Skin-warm, dry, acyanotic. Peeling of palmar area of bilateral feet.
Vascular-2+ pulses in all extremities.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days

| Lab | Units | 02/07/24
0535 | 02/06/24
0529 |
WBC	K/mcL	5.3	5.0
HGB	g/dL	**9.2***	**9.1***
MCV	fL	100	100
PLT	K/mcL	**446***	**472***
SODIUM	mmol/L	**131***	**132***
POTASSIUM	mmol/L	4.4	4.1
CHLORIDE	mmol/L	100	103
CO2	mmol/L	29	25
BUN	mg/dL	**20***	16
CREATININE	mg/dL	0.87	0.71
CALCIUM	mg/dL	8.8	8.8
GLUCOSE	mg/dL	94	103
ALBUMIN	g/dL	**3.3***	**3.3***
AST	Units/L	32	36
ALT	Units/L	21	26
BILIRUBIN TOTAL	mg/dL	0.2	0.2
ALK PHOS	Units/L	90	84

[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Speci
men: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain

	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#6]:
Assessment and Plan

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # Retropharyngeal and prevertebral fluid collection
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.

- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff -\
-> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid
- IR will not be able to drain the area.
- Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-) PICC line placed 1/30

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-)
-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of right palm, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy
Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
 # Bilateral leg edema
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg, Entresto
-IV Lasix x 3 doses. Resume oral Lasix afterwards.
-Strict I's and O's while on Lasix

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
- Fluid restriction to 2L daily
- Monitor serum sodium.

#Hypertension
-Carvedilol 3.125 mg p.o. twice daily and Entresto

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving

Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
\
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Elevated creatinine- resolved
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Resume Lasix 02/01/2024. Held Lasix given increased diuresis and hyponatremia

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

Resident: Emuejevuoke Anastasia Umukoro, DO

Attending: Martha Dalia Strauser, MD

Cosigned by Martha Strauser, MD at 02/08/2024 8:47 AM PST
Electronically signed by Emuejevuoke Umukoro, DO at 02/07/2024 1:02 PM PST
Electronically signed by Emuejevuoke Umukoro, DO at 02/07/2024 1:03 PM PST
Electronically signed by Martha Strauser, MD at 02/08/2024 8:47 AM PST

Howie Que, DPM - 02/07/2024 12:19 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/07/24 0918
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/07/24 0909
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• ben
zonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/07/24 0910 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/07/24 0910
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/06/24 1635
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Ani, Samuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/07/24 1135
• furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO, 40 mg at 02/07/24 0910
• gabapentin (NEURONTIN) capsule 100 mg, 100 mg, oral, TID, Umukoro, Emuejevuoke Anastasia, DO, 100 mg at 02/07/24 0910
• HYDROcodone-acetaminophen (NORCO) 10-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 tablet at 02/07/24 1135
• HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 1 tablet, 1 tablet, oral, Q4H PRN, Umukoro, Emuejevuoke Anastasia, DO
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/06/24 0239
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/07/24 1037
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/07/24 0459
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/06/24 1744
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/03/24 0200
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/06/24 1459 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Ani, Samuel, MD, 500 mg at 02/07/24 0910
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/07/24 0910
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/07/24 0909
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/06/24 2058

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	02/07/24 1135
BP:	113/74
Pulse:	89
Resp:	19
Temp:	36.4 °C (97.5 °F)
SpO2:	100%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:

Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossove
r on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine and abx therapy.
4. Nails debrided without incident.

[table#2]:
Results from last 7 days
Lab	Units	02/07/24
0535
SODIUM	mmol/L	**131***
POTASSIUM	mmol/L	4.4
CHLORIDE	mmol/L	100
CO2	mmol/L	29
BUN	mg/dL	**20***
CREATININE	mg/dL	0.87
GLUCOSE	mg/dL	94
CALCIUM	mg/dL	8.8

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/07/2412:22 PM

Electronically signed by Howie Que, DPM at 02/07/2024 12:25 PM PST
Electronically signed by Howie Que, DPM at 02/07/2024 3:08 PM PST

Martha Strauser, MD - 02/07/2024 8:40 AM PST
Formatting of this note might be different from the original.
****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
Known to me 55-year-old homeless female with history of heart failure with reduced ejection fraction of 25 to 35%, alcohol abuse, asthma, methamphetamine use disorder. Admitted on January 12, 2024 with shortness of breath. Found to have a large pleural

effusion with atelectasis and empyema. S cervical spine tatus post chest tube placement. Status post intubation due to acute hypoxic hypercarbic respiratory failure. During ICU stay developed A-fib with rapid ventricular response. Patient was extubated her chest tube was removed on January 21, 2024 after treatment with tPA DNase. Later on developed neck pain. Had an MRI of cervical spine which reveals cervical discitis/osteomyelitis from C4-C6 with retropharyngeal and prevertebral fluid collection. Evaluated by neurosurgery not recommending any procedure at the present time. Consulted by ID. Is to continue ceftriaxone for at least 8 weeks in conjunction with metronidazole for 2 weeks. Has been evaluated by ENT due to prevertebral fluid collection and they recommended follow-up with CAT scans. Was evaluated by IR who recommended no drainage. Status post PICC line on January 30. Has been on Rocephin since January 25.

Objective
[table#2]:
**OBJECTIVE **
Temp: 36.9 °C (98.4 °F)
Heart Rate: 101
Resp: 20
BP: 117/79
SpO2: 92 %
Temp (24hrs), Avg:37 °C (98.6 °F), Min:36.8 °C (98.2 °F), Max:37.4 °C (99.3 °F)

SpO2: 92 %
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND
Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs
[table#3]:
| **Results from last 7 days** |
| -
-- |
| Lab | Units | 02/07/24
0535 | 02/06/24
0529 | 02/05/24
0553 |
| SODIUM | mmol/L | **131*** | **132*** | **131*** |
| POTASSIUM | mmol/L | 4.4 | 4.1 | 4.5 |
| CHLORIDE | mmol/L | 100 | 103 | 98 |
| CO2 | mmol/L | 29 | 25 | 29 |
| BUN | mg/dL | **20*** | 16 | 15 |
| CREATININE | mg/dL | 0.87 | 0.71 | 0.75 |
| CALCIUM | mg/dL | 8.8 | 8.8 | 8.7 |
| BILIRUBIN TOTAL | mg/dL | 0.2 | 0.2 | **0.1*** |
| ALK PHOS | Units/L | 90 | 84 | 107 |
| ALT | Units/L | 21 | 26 | 22 |
| AST | Units/L | 32 | 36 | **39*** |
| GLUCOSE | mg/dL | 94 | 103 | 94 |

[table#4]:
Results from last 7 days
Lab	Units	02/07/24
0535	02/06/24
0529	02/05/24
0553
WBC	K/mcL	5.3	5.0	5.3
HGB	g/dL	**9.2***	**9.1***	**8.8***
HEMATOCRIT	%	**29.5***	**30.5***	**29.0***
PLT	K/mcL	**446***	**472***	**515***
NEUTROS PCT AUTO	%	44.2	42.6	49.7
LYMPHS PCT AUTO	%	41.7	42.5	35.5
MONOS PCT AUTO	%	9.2	10.5	10.2
EOS PCT AUTO	%	3.2	2.8	3.0

[table#5]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/28/2024 07:25 AM	130 (H)	70 - 125 mg/dL	Final
01/26/2024 12:06 PM	131 (H)	70 - 125 mg/dL	Final
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final

Imaging Studies
No results found.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 40 mg, oral, Daily
gabapentin, 100 mg, oral, TID
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current Infusion Medications:
dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 02/07/24 0107
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/06/24 0239
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/06/24 1640
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/03/24 0200
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/06/24 1459
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 02/06/24 2058
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/06/24 2058

Assessment/Plan
[table#6]:
**ASSESSMENT AND PLAN **

Elizabeth Tina Petruccio is a 59 y.o. year old female with history of alcohol and methamphetamine use disorder, dilated cardiomyopathy with ejection fraction of 25 to 35%. Admitted with shortness of breath and hypoxic hypercarbic respiratory failure requiring intubation. Found to have a right pleural effusion s/p chest tube treatment with tPA DNase. Later on developed neck pain found to have C4-C5 C5-C6 discitis osteomyelitis with prevertebral fluid collection which has been evaluated by ENT and IR. Per ID continue Rocephin to complete 8 weeks as well as metronidazole for 14 days.

-Cervical discitis with osteomy
elitis affecting C4-C5 C5-C6-continue Rocephin to complete 8 weeks of treatment. Continue ENT and ID recommendations.
-Status post acute hypoxic hypercarbic respiratory failure resolved. Status post intubation and extubation on January 14.
-Sepsis secondary to pneumococcus with right-sided empyema. The status post chest tube placement from January 13 to January 21.
-COPD-stable
-A-fib with rapid ventricular test follows with CHA2DS2-VASc 2 score of 2-patient decided for anticoagulation with Eliquis
-Weakness-PT OT
-Methamphetamine and tobacco abuse-resources in the community have been provided to patient
-Right ankle gout-colchicine
-Oral ulcers a status post Harrop's outbreak-pain much improved with acyclovir
-DVT prophylaxis Eliquis.
-Disposition-SNF
LOS: 26 days
**Disposition Plan: **Quarantined environment pending medical progression.

Code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 02/07/2024 12:25 PM PST

Howie Que, DPM - 02/06/2024 2:49 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/06/24 0824
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/06/24 0824
• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853

• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/06/24 0824 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/06/24 0824
• cefTRIAXone (ROCEPHIN) in steri
le water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/05/24 1636
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Ani, Samuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/06/24 1110
• [START ON 2/7/2024] furosemide (LASIX) tablet 40 mg, 40 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO
• gabapentin (NEURONTIN) capsule 100 mg, 100 mg, oral, TID, Umukoro, Emuejevuoke Anastasia, DO
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Ani, Samuel, MD, 0.5 mg at 02/06/24 0823
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/06/24 0239
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/05/24 2258
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/06/24 0440
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/05/24 1636
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/03/24 0200
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/02/24 2106 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Ani, Samuel, MD, 500 mg at 02/06/24 0824
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/06/24 0824
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Ani, Samuel, MD, 5 mg at 02/05/24 0429
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/06/24 0824
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/05/24 2313

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	02/06/24 1359
BP:	126/82
Pulse:	76
Resp:	18
Temp:	
SpO2:	

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on
clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/06/24
0529
SODIUM	mmol/L	**132***
POTASSIUM	mmol/L	4.1
CHLORIDE	mmol/L	103
CO2	mmol/L	25
BUN	mg/dL	16
CREATININE	mg/dL	0.71
GLUCOSE	mg/dL	103
CALCIUM	mg/dL	8.8

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/06/242:50 PM

Electronically signed by Howie Que, DPM at 02/06/2024 2:50 PM PST

Emuejevuoke Umukoro, DO - 02/06/2024 12:45 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amount of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also,

ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infection is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/6/2024: Patient is sitting comfortably in her chair. She complains of right shoulder pain, though the pain is not new. She also complains of persistent bilateral foot swelling. Denies a
ny other complaints.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 120/87 (BP Location: Left arm, Patient Position: Sitting) | Pulse 94 | Temp 37 °C (98.6 °F) (Oral) | Resp 20 | Ht 1.676 m (5' 6") | Wt 67 kg (147 lb 11.3 oz) | LMP (LMP Unknown) | SpO2 98% | Breastfeeding No | BMI 23.84 kg/m²
I/O: In: 660 [P.O.:660]
Out: 550 [Urine:550]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.** Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+2 pitting edema around ankles and bilateral feet
Skin-warm, dry, acyanotic. Peeling of bilateral palms, R > L-improving .
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/06/24
0529	02/05/24
0553
WBC	K/mcL	5.0	5.3
HGB	g/dL	**9.1***	**8.8***
MCV	fL	100	**101***
PLT	K/mcL	**472***	**515***
SODIUM	mmol/L	**132***	**131***
POTASSIUM	mmol/L	4.1	4.5
CHLORIDE	mmol/L	103	98
CO2	mmol/L	25	29
BUN	mg/dL	16	15
CREATININE	mg/dL	0.71	0.75
CALCIUM	mg/dL	8.8	8.7
GLUCOSE	mg/dL	103	94
ALBUMIN	g/dL	**3.3***	**3.2***
AST	Units/L	36	**39***
ALT	Units/L	26	22
BILIRUBIN TOTAL	mg/dL	0.2	**0.1***
ALK PHOS	Units/L	84	107

[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)

	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection
Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain

	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#6]:
Assessment and Plan

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # Retropharyngeal and prevertebral fluid collection.
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid
- IR will not be able to drain the area.
- Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-) PICC line placed 1/30

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-)
-Pulmonary team, sig
ned off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

 # Peeling/desquamation of right palm, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

 # Chronic right shoulder pain, likely radiculopathy

Patient has history of cervical stenosis noted on imaging. So, it is very likely that her shoulder pain is secondary to that.
-Scheduled gabapentin
-Monitor

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg.
-Entresto
-Strict I's and O's while on Lasix

 # Elevated creatinine- resolved
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Will resume Lasix 02/01/2024. Holding Lasix currently given increased diuresis and hyponatremia
-Continue losartan

#Hypertension
-Carvedilol 3.125 mg p.o. twice daily and Entresto

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
-Fluid restriction to 1.5 L daily
- Continue to monitor serum sodium.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Pati
ent was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-

Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

Resident: Emuejevuoke Anastasia Umukoro, DO

Attending: Martha Dalia Strauser, MD

Cosigned by Martha Strauser, MD at 02/06/2024 1:19 PM PST
Electronically signed by Emuejevuoke Umukoro, DO at 02/06/2024 1:03 PM PST
Electronically signed by Martha Strauser, MD at 02/06/2024 1:19 PM PST

Hai Shao, MD, PhD - 02/06/2024 11:14 AM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
No fever overnight, feeling well with no new complaints.

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 02/06/24 0823
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/06/24 0239
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/05/24 2258
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/03/24 0200
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/02/24 2106
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 02/05/24 0429
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/05/24 2313

Current Vital Signs:
[table#2]:
Vitals:
	02/06/24 0349	02/06/24 0430	02/06/24 0740	02/06/24 0823
BP:		135/90	116/78	

BP Location:		Left arm	Left arm	
Patient Position:		Sitting	Lying	
Pulse:		105	101	
Resp:		20	20	
Temp:		36.8 °C (98.2 °F)	36.7 °C (98.1 °F)	
TempSrc:		Oral	Oral	
SpO2:		94%	96%	96%
Weight:	67 kg (147 lb 11.3 oz)			
Height:				

Physical Exam
Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No tenderness. No ed
ema.
Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	02/06/24
0529	02/05/24
0553	02/04/24
0411
WBC	K/mcL	5.0	5.3	4.1
HGB	g/dL	**9.1***	**8.8***	**8.0***
HEMATOCRIT	%	**30.5***	**29.0***	**27.2***
PLT	K/mcL	**472***	**515***	**461***
NEUTROS PCT AUTO	%	42.6	49.7	44.3
LYMPHS PCT AUTO	%	42.5	35.5	38.5
MONOS PCT AUTO	%	10.5	10.2	10.8
EOS PCT AUTO	%	2.8	3.0	4.2

[table#4]:
Results from last 7 days

| Lab | Units | 02/06/24
0529 | 02/05/24
0553 | 02/04/24
0411 |
SODIUM	mmol/L	**132***	**131***	**131***
POTASSIUM	mmol/L	4.1	4.5	3.9
CHLORIDE	mmol/L	103	98	99
CO2	mmol/L	25	29	30
BUN	mg/dL	16	15	16
CREATININE	mg/dL	0.71	0.75	0.87
CALCIUM	mg/dL	8.8	8.7	8.7
BILIRUBIN TOTAL	mg/dL	0.2	**0.1***	**<0.1***
ALK PHOS	Units/L	84	107	81
ALT	Units/L	26	22	22
AST	Units/L	36	**39***	**38***
GLUCOSE	mg/dL	103	94	99

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final

| | Gram Stain | Many WBCs |
| | Gram Stain | No organisms seen |
| **Respiratory culture with stain** |
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean
catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

IMAGING:
IR picc line placement - us/fluoro guidance as needed

Result Date: 1/30/2024
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance. release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with

osteophytes. CRP is better but ESR still grossly elevated. D/w RN, continue rocephin (day 12/56) and flagyl (day 12/14). Picc line placement and discharge planning.

Dr. Koura will cover me till Feb. 18th on prn basis.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 02/06/2024 11:19 AM PST

Martha Strauser, MD - 02/06/2024 10:20 AM PST

Formatting of this note might be different from the original.

****[image]****

Hospitalist Progress Note

Subjective

[table#1]:

**SUBJECTIVE **

Known to me 55-year-old homeless female with history of heart failure with reduced ejection fraction of 25 to 35%, alcohol abuse, asthma, methamphetamine use disorder. Admitted on January 12, 2024 with shortness of breath. Found to have a large pleural effusion with atelectasis and empyema. S cervical spine tatus post chest tube placement. Status post intubation due to acute hypoxic hypercarbic respiratory failure. During ICU stay developed A-fib with rapid ventricular response. Patient was extubated her chest tube was removed on January 21, 2024 after treatment with tPA DNase. Later on developed neck pain. Had an MRI of cervical spine which reveals cervical discitis/osteomyelitis from C4-C6 with retropharyngeal and prevertebral fluid collection. Evaluated by neurosurgery not recommending any procedure at the present time. Consulted by ID. Is to continue ceftriaxone for at least 8 weeks in conjunction with metronidazole for 2 weeks. Has been evaluated by ENT due to prevertebral fluid collection and they recommended follow-up with CAT scans. Was evaluated by IR who recommended no drainage. Status post PICC line on January 30. Has been on Rocephin since January 25.

Objective

[table#2]:

**OBJECTIVE **

Temp: 36.7 °C (98.1 °F)
Heart Rate: 101
Resp: 20
BP: 116/78
SpO2: 96 %
Temp (24hrs), Avg:36.7 °C (98.1 °F), Min:36.6 °C (97.9 °F), Max:36.8 °C (98.2 °F)

SpO2: 96 %
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated
, SNTND
Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs

[table#3]:

Results from last 7 days
Lab	Units	02/06/24
0529	02/05/24
0553	02/04/24
0411
SODIUM	mmol/L	**132***	**131***	**131***
POTASSIUM	mmol/L	4.1	4.5	3.9
CHLORIDE	mmol/L	103	98	99
CO2	mmol/L	25	29	30
BUN	mg/dL	16	15	16
CREATININE	mg/dL	0.71	0.75	0.87
CALCIUM	mg/dL	8.8	8.7	8.7
BILIRUBIN TOTAL	mg/dL	0.2	**0.1***	**<0.1***

ALK PHOS	Units/L	84	107	81
ALT	Units/L	26	22	22
AST	Units/L	36	**39***	**38***
GLUCOSE	mg/dL	103	94	99

[table#4]:
Results from last 7 days
Lab	Units	02/06/24
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0553	02/04/24
0411
WBC	K/mcL	5.0	5.3	4.1
HGB	g/dL	**9.1***	**8.8***	**8.0***
HEMATOCRIT	%	**30.5***	**29.0***	**27.2***
PLT	K/mcL	**472***	**515***	**461***
NEUTROS PCT AUTO	%	42.6	49.7	44.3
LYMPHS PCT AUTO	%	42.5	35.5	38.5
MONOS PCT AUTO	%	10.5	10.2	10.8
EOS PCT AUTO	%	2.8	3.0	4.2

[table#5]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/28/2024 07:25 AM	130 (H)	70 - 125 mg/dL	Final
01/26/2024 12:06 PM	131 (H)	70 - 125 mg/dL	Final
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final

Imaging Studies
No results found.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current Infusion Medications:
dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN

HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 02/06/24 0823
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/06/24 0239
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/05/24 2258
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/03/24 0200
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/02/24 2106
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 02/05/24 0429
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/05/24 2313

Assessment/Plan
[table#6]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with history of alcohol and methamphetamine use disorder, dilated cardiomyopathy with ejection fraction of 25 to 35%. Admitted with shortness of breath and hypoxic hypercarbic respiratory failure requiring intubation. Found to have a right pleural effusion s/p chest tube treatment with tPA DNase. Later on developed neck pain found to have C4-C5 C5-C6 discitis osteomyelitis with prevertebral fluid collection which has been evaluated by ENT and IR. Per ID continue R
ocephin to complete 8 weeks as well as metronidazole for 14 days.

-Cervical discitis with osteomyelitis affecting C4-C5 C5-C6-continue Rocephin to complete 8 weeks of treatment. Continue ENT and ID recommendations.
-Status post acute hypoxic hypercarbic respiratory failure resolved. Status post intubation and extubation on January 14.
-Sepsis secondary to pneumococcus with right-sided empyema. The status post chest tube placement from January 13 to January 21.
-COPD-stable
-A-fib with rapid ventricular test follows with CHA2DS2-VASc 2 score of 2-patient decided for anticoagulation with Eliquis
-Weakness-PT OT
-Methamphetamine and tobacco abuse-resources in the community have been provided to patient
-Right ankle gout-colchicine
-Oral ulcers a status post Harrop's outbreak-pain much improved with acyclovir
-DVT prophylaxis Eliquis.
-Disposition-SNF
LOS: 25 days
**Disposition Plan: **Quarantined environment pending medical progression.

Code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 02/06/2024 1:15 PM PST

Your Scripps Health Team - 02/06/2024 8:01 AM PST

Formatting of this note might be different from the original.
Admitted for sob & sepsis 2/2 empyema. S/p chest tube placed 1/13 & removed 1/21. IV/PO abx. S/p intubation 1/13 & extubation 1/14. Hx of homelessness, etoh, & meth use. Walked 400'. Stays w/ son Anthony in Huntington Beach shelters or travels in MX, indp pta. Dtr Erika lives in NJ. Medi-cal only--HMO application sent 1/22. Not candidate for alt bed. Dcp: SNF for IV abx for total 56 days. MCAL call list. PASRR L1 done.
**SHEILA MARIE P. **
RN/CM/BSN
SCRIPPS MERCY CHULA VISTA

Electronically signed by Your Scripps Health Team at 02/06/2024 11:41 AM PST
Electronically signed by Your Scripps Health Team at 02/06/2024 11:46 AM PST
Electronically signed by Your Scripps Health Team at 02/06/2024 11:56 AM PST

Emuejevuoke Umukoro, DO - 02/05/2024 3:51 PM PST

Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. CXR revealed large right pleural effusion with subjacent atelectasis, hyperinflated left lung. She had to be intubated for worsening acute hypoxic and hypercapnic respiratory failure and admitted to the ICU. Patient extubated 1/14 and transferred out of the ICU. She was also found to have empyema. Patient required chest tube placement, eventually removed 1/21/2024. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem and amio gtt. she was started on Eliquis 5 mg on 2/3/2024 given CHA2DS2-VASc score of 2 after discussion of risk and benefits.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She received tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024 and chest tube removed 1/21/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration.

She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings consistent with C4-C5 and C5-C6 discitis and osteomyelitis and prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited but showed a large amount of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also, ENT Dr. Hauff was consulted and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. She was recommended follow-up with neurosurgery once her infecti
on is treated, given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.

2/5/2024: No acute events overnight. Patient resting and on 2 L nasal cannula. She reports hip pain and foot pain due to the position of the bed. Also reports an episode of RUQ pain, but prior ultrasound 1/13/2024 was unremarkable. No other acute complaints.

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP 111/80 (BP Location: Left arm, Patient Position: Sitting) | Pulse 110 | Temp 37.4 °C (99.3 °F) (Oral) | Resp 18 | Ht 1.676 m (5' 6") | Wt 67.2 kg (148 lb 2.4 oz) | LMP (LMP Unknown) | SpO2 94% | Breastfeeding No | BMI 23.91 kg/m^2
I/O: In: 430 [P.O.:410; I.V.:20]
Out: 1525 [Urine:1525]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.** Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+2 pitting edema around ankles and bilateral feet
Skin-warm, dry, acyanotic. Peeling of bilateral palms, R > L-improving .
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/05/24
0553	02/04/24
0411

WBC	K/mcL	5.3	4.1
HGB	g/dL	**8.8***	**8.0***
MCV	fL	**101***	**102***
PLT	K/mcL	**515***	**461***
SODIUM	mmol/L	**131***	**131***
POTASSIUM	mmol/L	4.5	3.9
CHLORIDE	mmol/L	98	99
CO2	mmol/L	29	30
BUN	mg/dL	15	16
CREATININE	mg/dL	0.75	0.87
CALCIUM	mg/dL	8.7	8.7
GLUCOSE	mg/dL	94	99
ALBUMIN	g/dL	**3.2***	**2.9***
AST	Units/L	**39***	**38***
ALT	Units/L	22	22
BILIRUBIN TOTAL	mg/dL	**0.1***	**<0.1***
ALK PHOS	Units/L	107	81

[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, righ
t; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value

	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#6]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # retropharyngeal and prevertebral fluid collection.
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis and persistent prevertebral fluid

- IR will not be able to drain the area.
- Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-) PICC line placed 1/30

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu a
nd COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-)
-Pulmonary team, signed off 1/30/24, open to re-consultation if needed, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

#Peeling/desquamation of right palm, improving
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amio and diltiazem gtt.
Given patient's low EF, had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ. However she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Discussed with patient that her A-fib may have just been in the setting of acute illness, but it is hard to tell if she could have paroxysmal A-fib. At this time, she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.
-Eliquis 5 mg BID

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg.
-Entresto
-Strict I's and O's while on Lasix

#**Uptrending Creatinine- **normalized
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Will resume Lasix 02/01/2024. Holding Lasix currently given increased diuresis and hyponatremia
-Continue losartan

#Hypertension
-Carvedilol 3.125 mg p.o. twice daily and Entresto

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **

 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Hyponatremia, likely SIADH d/t lung pathology
1/20/2024: Urine sodium of 17, urine osmolality 152, serum osmolality 265. Likely due to decreased intake or losses not related to kidney disease
-Fluid restriction to 1.5 L daily
- Continue to monitor serum sodium.

 # Housing instability
**# Su
bstance use disorder (methamphetamine)**
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance. Case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

Resident: Emuejevuoke Anastasia Umukoro, DO

Attending: Martha Dalia Strausser, MD

Cosigned by Martha Strauser, MD at 02/06/2024 10:20 AM PST
Electronically signed by Emuejevuoke Umukoro, DO at 02/05/2024 4:11 PM PST
Electronically signed by Martha Strauser, MD at 02/06/2024 10:20 AM PST

Howie Que, DPM - 02/05/2024 3:15 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/05/24 0849
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/05/24 0848

• artificial tears ophthalmic solution 1 drop, 1 drop, each eye, 4x Daily PRN, de Four, Amanda R, MD, 1 drop at 02/04/24 2116
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/05/24 0906 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/05/24 0848
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/04/24 1620
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Ani, Sa
muel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/05/24 1259
• furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 02/05/24 0848
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Ani, Samuel, MD, 0.5 mg at 02/05/24 0848
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/04/24 2350
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/05/24 0953
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/05/24 0430
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/04/24 1619
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/03/24 0200
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/02/24 2106 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Ani, Samuel, MD, 500 mg at 02/05/24 0847
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/05/24 0847
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Ani, Samuel, MD, 5 mg at 02/05/24 0429
• sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet, 1 tablet, oral, BID, de Four, Amanda R, MD, 1 tablet at 02/05/24 0848
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/01/24 0143

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	02/05/24 1134
BP:	111/80
Pulse:	110
Resp:	18
Temp:	
SpO2:	94%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout

2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, an
d Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/05/24
0553
SODIUM	mmol/L	**131***
POTASSIUM	mmol/L	4.5
CHLORIDE	mmol/L	98
CO2	mmol/L	29
BUN	mg/dL	15
CREATININE	mg/dL	0.75
GLUCOSE	mg/dL	94
CALCIUM	mg/dL	8.7

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/05/243:16 PM

Electronically signed by Howie Que, DPM at 02/05/2024 3:17 PM PST

Hai Shao, MD, PhD - 02/05/2024 1:00 PM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
No fever overnight, feeling well with no new complaints.

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN

Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 02/05/24 0848
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/04/24 2350
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/05/24 0953
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/03/24 0200
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/02/24 2106
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 02/05/24 0429
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/01/24 0143

Current Vital Signs:
[table#2]:
Vitals:
	02/05/24 0729	02/05/24 0953	02/05/24 1120	02/05/24 1134
BP:	110/77			111/80
BP Location:	Left arm			Left arm
Patient Position:	Sitting			Sitting
Pulse:	114	130	(P) 109	110
Resp:	20	16		18
Temp:	37.4 °C (99.3 °F)			
TempSrc:	Oral			
SpO2:	92%	97%	(P) 94%	94%
Weight:				
Height:				

Physical Exam
Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, cre
pitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No tenderness. No edema.
Skin:

General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	02/05/24
0553	02/04/24
0411	02/03/24
0542
WBC	K/mcL	5.3	4.1	4.5
HGB	g/dL	**8.8***	**8.0***	**8.0***
HEMATOCRIT	%	**29.0***	**27.2***	**26.4***
PLT	K/mcL	**515***	**461***	**496***
NEUTROS PCT AUTO	%	49.7	44.3	52.8
LYMPHS PCT AUTO	%	35.5	38.5	32.7
MONOS PCT AUTO	%	10.2	10.8	8.5
EOS PCT AUTO	%	3.0	4.2	4.0

[table#4]:
Results from last 7 days
Lab	Units	02/05/24
0553	02/04/24
0411	02/03/24
0542
SODIUM	mmol/L	**131***	**131***	**131***
POTASSIUM	mmol/L	4.5	3.9	4.3
CHLORIDE	mmol/L	98	99	102
CO2	mmol/L	29	30	29
BUN	mg/dL	15	16	**19***
CREATININE	mg/dL	0.75	0.87	0.82
CALCIUM	mg/dL	8.7	8.7	8.6
BILIRUBIN TOTAL	mg/dL	**0.1***	**<0.1***	**0.1***
ALK PHOS	Units/L	107	81	76
ALT	Units/L	22	22	24
AST	Units/L	**39***	**38***	**42***
GLUCOSE	mg/dL	94	99	99

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM

	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growt
h Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2

	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

IMAGING:
IR picc line placement - us/fluoro guidance as needed

Result Date: 1/30/2024
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance. release

CT cervical spine with contrast

Result Date: 1/29/2024
IMPRESSION: 1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 1/29/2024
IMPRESSION: Negative for deep vein thrombosis in both lower extremities. release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with osteophytes. CRP is better but ESR still grossly elevated. D/w RN, continue rocephin (day 11/56) and flagyl (day 11/14). Picc line placement and discharge planning.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 02/05/2024 1:00 PM PST

Martha Strauser, MD - 02/05/2024 12:31 PM PST
Formatting of this note might be different from the original.
****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
Known to me 55-year-old homeless female with history of heart failure with reduced ejection fraction of 25 to 35%, alcohol abuse, asthma, methamphetamine use disorder. Admitted on January 12, 2024 with shortness of breath. Found to have a large pleural effusion with atelectasis and empyema. S cervical spine tatus post chest tube placement. Status post intubation due to acute hypoxic hypercarbic respiratory failure. During ICU stay developed A-fib with rapid ventricular response. Patient was extubated her chest tube was removed on January 21, 2024 after treatment with tPA DNase. Later on developed neck pain. Had an MRI of cervical spine which reveals cervical discitis/osteomyelitis from C4-C6 with retropharyngeal and prevertebral fluid collection. Evaluated by neurosurgery not recommending any procedure at the
present time. Consulted by ID. Is to continue ceftriaxone for at least 8 weeks in conjunction with metronidazole for 2 weeks. Has been evaluated by ENT due to prevertebral fluid collection and they recommended follow-up with CAT scans. Was evaluated by IR who recommended no drainage. Status post PICC line on January 30. Has been on Rocephin since January 25.

Objective
[table#2]:
**OBJECTIVE **
Temp: 37.4 °C (99.3 °F)
Heart Rate: 110
Resp: 18
BP: 111/80
SpO2: 94 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 0 L/min
Temp (24hrs), Avg:37.1 °C (98.8 °F), Min:36.9 °C (98.4 °F), Max:37.4 °C (99.3 °F)

SpO2: 94 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 0 L/min
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND
Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs
[table#3]:
Results from last 7 days
Lab	Units	02/05/24
0553	02/04/24
0411	02/03/24
0542
SODIUM	mmol/L	**131***	**131***	**131***
POTASSIUM	mmol/L	4.5	3.9	4.3
CHLORIDE	mmol/L	98	99	102
CO2	mmol/L	29	30	29
BUN	mg/dL	15	16	**19***
CREATININE	mg/dL	0.75	0.87	0.82
CALCIUM	mg/dL	8.7	8.7	8.6
BILIRUBIN TOTAL	mg/dL	**0.1***	**<0.1***	**0.1***
ALK PHOS	Units/L	107	81	76
ALT	Units/L	22	22	24
AST	Units/L	**39***	**38***	**42***
GLUCOSE	mg/dL	94	99	99

[table#4]:
Results from last 7 days
Lab	Units	02/05/24
0553	02/04/24
0411	02/03/24
0542
WBC	K/mcL	5.3	4.1	4.5
HGB	g/dL	**8.8***	**8.0***	**8.0***
HEMATOCRIT	%	**29.0***	**27.2***	**26.4***
PLT	K/mcL	**515***	**461***	**496***
NEUTROS PCT AUTO	%	49.7	44.3	52.8
LYMPHS PCT AUTO	%	35.5	38.5	32.7
MONOS PCT AUTO	%	10.2	10.8	8.5
EOS PCT AUTO	%	3.0	4.2	4.0

[table#5]:
Glucose Blood, POC

Date/Time	Value	Ref Range	Status
01/28/2024 07:25 AM	130 (H)	70 - 125 mg/dL	Final
01/26/2024 12:06 PM	131 (H)	70 - 125 mg/dL	Final
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final

Imaging Studies
No results found.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sacubitriL-valsartan, 1 tablet, oral, BID
sennosides-docusate sodium, 1 tablet, oral, BID

Current Infusion Medications:
dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, 4x Daily PRN, 1 drop at 02/04/24 2116
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 02/05/24 0848
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/04/24 2350
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/05/24 0953
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/
03/24 0200
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/02/24 2106
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 02/05/24 0429
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/01/24 0143

Assessment/Plan
[table#6]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with history of alcohol and methamphetamine use disorder, dilated cardiomyopathy with ejection fraction of 25 to 35%. Admitted with shortness of breath and hypoxic hypercarbic respiratory failure requiring intubation. Found to have a right pleural effusion s/p chest tube treatment with tPA DNase. Later on developed neck pain found to have C4-C5 C5-C6 discitis osteomyelitis with prevertebral fluid collection which has been evaluated by ENT and IR. Per ID continue Rocephin to complete 8 weeks as well as metronidazole for 14 days.

-Cervical discitis with osteomyelitis affecting C4-C5 C5-C6-continue Rocephin to complete 8 weeks of treatment. Continue ENT and

ID recommendations.
-Status post acute hypoxic hypercarbic respiratory failure resolved. Status post intubation and extubation on January 14.
-Sepsis secondary to pneumococcus with right-sided empyema. The status post chest tube placement from January 13 to January 21.
-COPD-stable
-A-fib with rapid ventricular test follows with CHA2DS2-VASc 2 score of 2-patient decided for anticoagulation with Eliquis
-Weakness-PT OT
-Methamphetamine and tobacco abuse-resources in the community have been provided to patient
-Right ankle gout-colchicine
-Oral ulcers a status post Harrop's outbreak-pain much improved with acyclovir
-DVT prophylaxis Eliquis.
-Disposition-SNF
LOS: 24 days
**Disposition Plan: **Quarantined environment pending medical progression.

Code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 02/05/2024 12:40 PM PST

Hai Shao, MD, PhD - 02/04/2024 2:45 PM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
No fever overnight, feeling well with no new complaints.

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:
ammonium lactate, , topical, BID
apixaban, 5 mg, oral, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
losartan, 25 mg, oral, At bedtime
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 02/04/24 0835
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/03/24 2214
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/04/24 0547
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/03/24 0200
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/02/24 2106
And

metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 02/03/24 2214
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/01/24 0143

Current Vital Signs:
[table#2]:
Vitals:
	02/04/24 0547	02/04/24 0738	02/04/24 0800	02/04/24 1142
BP:		108/77		131/89
BP Location:		Left arm		Left arm
Patient Position:		Sitting		Sitting
Pulse:		124		91
Resp:		20		19
Temp:		36.7 °C (98.1 °F)		36.7 °C (98.1 °F)
TempSrc:		Oral		Oral
SpO2:	94%	95%	97%	97%
Weight:				
Height:				

Physical Exam
Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No tenderness. No edema.
Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	02/04/24
0411	02/03/24
0542	02/02/24
0521

WBC	K/mcL	4.1	4.5	5.3
HGB	g/dL	**8.0***	**8.0***	**7.9***
HEMATOCRIT	%	**27.2***	**26.4***	**26.2***
PLT	K/mcL	**461***	**496***	**513***
NEUTROS PCT AUTO	%	44.3	52.8	44.9
LYMPHS PCT AUTO	%	38.5	32.7	39.9
MONOS PCT AUTO	%	10.8	8.5	8.6
EOS PCT AUTO	%	4.2	4.0	4.4

[table#4]:
Results from last 7 days
Lab	Units	02/04/24
0411	02/03/24
0542	02/02/24
0521
SODIUM	mmol/L	**131***	**131***	**130***
POTASSIUM	mmol/L	3.9	4.3	4.5
CHLORIDE	mmol/L	99	102	100
CO2	mmol/L	30	29	30
BUN	mg/dL	16	**19***	**19***
CREATININE	mg/dL	0.87	0.82	0.87
CALCIUM	mg/dL	8.7	8.6	**8.3***
BILIRUBIN TOTAL	mg/dL	**<0.1***	**0.1***	**<0.1***
ALK PHOS	Units/L	81	76	71
ALT	Units/L	22	24	19
AST	Units/L	**38***	**42***	28
GLUCOSE	mg/dL	99	99	88

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01
AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid

Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

IMAGING:
IR picc line placement - us/fluoro guidance as needed

Result Date: 1/30/2024
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance. release

CT cervical spine with contrast

Result Date: 1/29/2024
IMPRESSION: 1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 1/29/2024
IMPRESSION: Negative for deep vein thrombosis in both lower extremities. release

X-ray chest 2 views with pa/ap and lateral

Result Date: 1/29/2024
IMPRESSION: No significant change from prior. release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with osteophytes. CRP
is better but ESR still grossly elevated. D/w RN, continue rocephin (day 10/56) and flagyl (day 10/14). Picc line placement and discharge planning.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 02/04/2024 2:46 PM PST

Howie Que, DPM - 02/04/2024 1:56 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD

• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/04/24 0835
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/04/24 0833
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/04/24 0833 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/04/24 0833
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/03/24 1522
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Ani, Samuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/04/24 1209
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 02/01/24 0908
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Ani, Samuel, MD, 0.5 mg at 02/04/24 0835
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PR
N, Ani, Samuel, MD, 25 mg at 02/03/24 2214
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/04/24 0547
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/04/24 0834
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/03/24 1638
• losartan (COZAAR) tablet 25 mg, 25 mg, oral, At bedtime, de Four, Amanda R, MD, 25 mg at 02/03/24 2215
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/03/24 0200
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/02/24 2106 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Ani, Samuel, MD, 500 mg at 02/04/24 0833
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/04/24 0833
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Ani, Samuel, MD, 5 mg at 02/03/24 2214
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/01/24 0143

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	02/04/24 1142
BP:	131/89
Pulse:	91
Resp:	19
Temp:	36.7 °C (98.1 °F)
SpO2:	97%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:

1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/04/24
0411
SODIUM	mmol/L	**131***
POTASSIUM	mmol/L	3.9
CHLORIDE	mmol/L	99
CO2	mmol/L	30
BUN	mg/dL	16
CREATININE	mg/dL	0.87
GLUCOSE	mg/dL	99
CALCIUM	mg/dL	8.7

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)

Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/04/241:56 PM

Electronically signed by Howie Que, DPM at 02/04/2024 1:57 PM PST

Amanda de Four, MD - 02/04/2024 11:10 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF EF of less than 25%, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Patient also required chest tube placement that was eventually removed after adequate improvement in empyema. Did not require CT surgery intervention. She was also noted to have A-fib RVR requiring diltiazem gtt. has since been started on Eliquis 5 mg p.o. twice daily given CHA2DS2-VASc score of 2 after discussion of risk and benefits. Patient extubated 1/14 and transferred out of the ICU.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She then started receiving tPA's and DNAse through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

*She was having persistent neck pain and had a CT of her cervical spine with contrast on 1/29/2024 that showed findings

consistent with C4-C5 discitis and osteomyelitis as well as C5-C6 discitis osteomyelitis. Patient had persistent prevertebral fluid. Patient had MRI on 1/26 with contrast that was very limited due to extensive motion but showed a large amount of partly enhancing posterior nasopharyngeal, prevertebral and anterior mediastinal fluid and edema. She was started back on antibiotics with IV Rocephin and p.o. Flagyl to take for total of 56 days for vertebral osteomyelitis. Also ENT Dr. Hauff was consulted due to fluid seen in nasopharyngeal prevertebral anterior mediastinal spaces and recommended conservative management with antibiotics at this time. Patient has no neurologic deficits and no symptoms of neuropathy. I have recommended she just follow-up once her infection is treated with neurosurgeon given her evidence of degenerative disc disease and cervical spine with moderate central canal stenosis.*

2/04/2024
Called patient's daughter this morning at her request updated her on her current clinical course. Patient is doing well up. Currently awaiting placement. Discussed with her starting anticoagulation for her A-fib she says she thinks she may have had a diagnosis of this before but cannot be sure. Anticoagulation started yesterday with Eliquis

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#2]:
Physical Exam
BP (!) 147/91 (BP Location: Left arm, Patient Position: Lying) | Pulse 103 | Temp 37.1 °C (98.8 °F) (Oral) | Resp 18 | Ht 1.676 m (5' 6") | Wt (P) 66.4 kg (146 lb 6.2 oz) | LMP (LMP Unknown) | SpO2 93% | Breastfeeding No | BMI (P) 23.63 kg/m²
I/O: In: 310 [P.O.:290; I.V.:20]
Out: 800 [Urine:800]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+2 pitting edema around ankles and bilateral feet
Skin-warm, dry, acyanotic. Peeling of bilateral palms, R > L-improving .
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#3]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#4]:
Results from last 7 days
Lab	Units	02/04/24
0411	02/03/24
0542
WBC	K/mcL	4.1	4.5
HGB	g/dL	**8.0***	**8.0***
MCV	fL	**102***	**101***
PLT	K/mcL	**461***	**496***
SODIUM	mmol/L	**131***	**131***
POTASSIUM	mmol/L	3.9	4.3
CHLORIDE	mmol/L	99	102
CO2	mmol/L	30	29
BUN	mg/dL	16	**19***
CREATININE	mg/dL	0.87	0.82
CALCIUM	mg/dL	8.7	8.6
GLUCOSE	mg/dL	99	99
ALBUMIN	g/dL	**2.9***	**3.0***
AST	Units/L	**38***	**42***
ALT	Units/L	22	24
BILIRUBIN TOTAL	mg/dL	**<0.1***	**0.1***
ALK PHOS	Units/L	81	76

[table#5]:
Recent Results (from the past 720 hour(s))

| **Influenza A/B** |
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.

| **MRSA Surveillance** |
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#6]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # retropharyngeal and prevertebral fluid collection.
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
-Recommended outpatient neurosurgery follow-up or orthopedic/spine surgery once patient has been treated for her spinal osteo-.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast
-CT cervical spine with contrast performed 01/29 showed . Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI
- IR will not be able to drain the area.
- Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-) Peak line placed 1/30

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to pneumococcal pneumonia with right-sided empyema status post chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-)
-Pulmonary team, signed off 1/30/24, open to re-consultation if need be, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

#Peeling/desquamation of right palm-improving with current cream.
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1

year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

**# A
-fib RVR (CHA2DS2-VASc score of 2)**
S/p amnio gtt.
-Given patient's low ejection fraction had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ however she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2. Did discuss with patient that her A-fib may have just been in the setting of acute illness but it is hard to tell if she could have paroxysmal A-fib. At this time she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg.
-Start patient on Entresto
-Strict I's and O's while on Lasix

#**Uptrending Creatinine- **normalized
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Will resume Lasix 02/01/2024. Holding Lasix currently given increased diuresis and hyponatremia
-Continue losartan

#Hypertension
-Carvedilol 3.125 mg p.o. twice daily and Entresto

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Hyponatremia, likely SIADH d/t lung pathology-135 -> 130
-Fluid restriction to 1.5 L daily
-Is/Os -730 ml in the last 24 hours. Total intake of 220 mls.
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265, 01/20/2024.
- Continue to monitor serum sodium.
-.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance, case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

Amanda R De Four, MD

Electronically signed by Amanda de Four, MD at 02/04/2024 9:22 PM PST

Hai Shao, MD, PhD - 02/03/2024 1:59 PM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
No fever overnight, feeling well with no new complaints.

Current Antibiotics
[table#1]:
| No current outpatient medications on file. |

| --- |

Current Scheduled Medications:
ammonium lactate, , topical, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
enoxaparin, 40 mg, subcutaneous, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
losartan, 25 mg, oral, At bedtime
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 02/02/24 2251
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 02/02/24 0933
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/02/24 1624
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/03/24 0847
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/03/24 0200
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/02/24 2106
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
naproxen, 500 mg, BID PRN, 500 mg at 02/03/24 0653
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 02/02/24 2122
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/01/24 0143

Current Vital Signs:
[table#2]:
Vitals:
	02/03/24 0700	02/03/24 0806	02/03/24 0941	02/03/24 1117
BP:	117/70	117/77		119/74
BP Location:	Left arm	Left arm		Left arm
Patient Position:	Sitting			Sitting
Pulse:	95	94	87	89

Resp:	20		18	18
Temp:	36.8 °C (98.2 °F)			37 °C (98.6 °F)
TempSrc:	Oral			Oral
SpO2:	91%		94%	94%
Weight:				
Height:				

Physical Exam
Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No tenderness. No edema.
Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	02/03/24
0542	02/02/24
0521	02/01/24
0545
WBC	K/mcL	4.5	5.3	5.2
HGB	g/dL	**8.0***	**7.9***	**7.5***
HEMATOCRIT	%	**26.4***	**26.2***	**24.6***
PLT	K/mcL	**496***	**513***	**573***
NEUTROS PCT AUTO	%	52.8	44.9	44.4
LYMPHS PCT AUTO	%	32.7	39.9	39.6
MONOS PCT AUTO	%	8.5	8.6	9.8
EOS PCT AUTO	%	4.0	4.4	4.2

[table#4]:
Results from last 7 days
Lab	Units	02/03/24
0542 | 02/02/24
0521 | 02/01/24

0545 |
SODIUM	mmol/L	**131***	**130***	**135***
POTASSIUM	mmol/L	4.3	4.5	4.2
CHLORIDE	mmol/L	102	100	102
CO2	mmol/L	29	30	**36***
BUN	mg/dL	**19***	**19***	17
CREATININE	mg/dL	0.82	0.87	0.70
CALCIUM	mg/dL	8.6	**8.3***	**8.3***
BILIRUBIN TOTAL	mg/dL	**0.1***	**<0.1***	**<0.1***
ALK PHOS	Units/L	76	71	71
ALT	Units/L	24	19	19
AST	Units/L	**42***	28	28
GLUCOSE	mg/dL	99	88	90

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain

	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collect
ion Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

IMAGING:
IR picc line placement - us/fluoro guidance as needed

Result Date: 1/30/2024
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance. release

CT cervical spine with contrast

Result Date: 1/29/2024
IMPRESSION: 1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 1/29/2024
IMPRESSION: Negative for deep vein thrombosis in both lower extremities. release

X-ray chest 2 views with pa/ap and lateral

Result Date: 1/29/2024
IMPRESSION: No significant change from prior. release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)

•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with osteophytes. CRP is better but ESR still grossly elevated. D/w RN, continue rocephin (day 9/56) and flagyl (day 9/14). Picc line placement and discharge planning.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 02/03/2024 2:03 PM PST

Howie Que, DPM - 02/03/2024 1:55 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, r
ectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/04/24 0835
• apixaban (ELIQUIS) tablet 5 mg, 5 mg, oral, BID, de Four, Amanda R, MD, 5 mg at 02/04/24 0833
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/04/24 0833 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 2251
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/04/24 0833
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/03/24 1522
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Ani, Samuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/04/24 1209

• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 02/01/24 0908
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Ani, Samuel, MD, 0.5 mg at 02/04/24 0835
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/03/24 2214
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/04/24 0547
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/04/24 0834
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/03/24 1638
• losartan (COZAAR) tablet 25 mg, 25 mg, oral, At bedtime, de Four, Amanda R, MD, 25 mg at 02/03/24 2215
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/03/24 0200
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/02/24 2106 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Ani, Samuel, MD, 500 mg at 02/04/24 0833
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/04/24 0833
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Ani, Samuel, MD, 5 mg at 02/03/24 2214
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/01/24 0143

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	02/04/24 1142
BP:	131/89
Pulse:	91
Resp:	19
Temp:	36.7 °C (98.1 °F)
SpO2:	97%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/04/24

0411 |
SODIUM	mmol/L	**131***
POTASSIUM	mmol/L	3.9
CHLORIDE	mmol/L	99
CO2	mmol/L	30
BUN	mg/dL	16
CREATININE	mg/dL	0.87
GLUCOSE	mg/dL	99
CALCIUM	mg/dL	8.7

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/04/241:55 PM

Electronically signed by Howie Que, DPM at 02/04/2024 1:56 PM PST

Amanda de Four, MD - 02/03/2024 10:47 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She then started receiving tPA's and dornase through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

***1/23/2024: ***Complaint of bilateral neck pain with movement, R>L. This is chronic and has been ongoing for many years, she believes it is headache vs migraine, but can clearly trace the pain towards the nape of the neck.
1/24/24: Colchicine dose decreased, slight improvement in foot pain. Medically cleared
for discharge to SNF, but her insurance is not being accepted by SNFs.
1/25/24: Severe headache, mostly occipital and seems to be of cervical origin although cannot rule out other causes of headache. Fluctuating, no clear pattern or triggers. No fever, no neck rigidity. Colchicine discontinued.
1/26/24: MRI showed discitis and prevertebral fluid collection. Case discussed with ID - Dr. Shao, ENT - Dr. Hauff and IR.
**1/27/24: **ENT recommended CT w/ contrast if no improvement. Restart CHF meds and add IV lasix.
Skin peeling/desquamation over the hands only. Patient reported extensive work up in NJ for autoimmune diseases.

1/29/2024: No acute events overnight. Patient has no acute complaints today. She does not answer if she has neck pain today and is hyper focused on getting her records scanned. Will scan her prior records in our system.

1/30/2024
Patient seen and examined at bedside. No acute events overnight. Continues to be afebrile. Currently on room air satting at 97%. Net +545 in the past 24 hours. Normal WBC. Creatinine uptrending after CT with contrast. CT cervical spine with contrast performed 01/29 showed Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. Vascular ultrasound of lower extremities performed 01/29/2024 showed no intramural thrombus. chest x-ray performed 01/29 showed Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.

01/31/24
No acute events overnight, BP of 146/93 and SpO2 of 87 noted at 0 349 today, Currently 95% on room air. Net -1 L in the past 24 hours. losartan and Lasix held due to increased creatinine 01/30/2024, and patient currently on Coreg 3.125 mg twice daily. Thoracentesis not performed by pulmonary, Dr. Jones, 01/30/2024, because of low vol of pleural effusion.

02/01/2024
Reported anxiety overnight which responded to medication. Patient downgraded to MedSurg on 01/31/2024. No new complaints and vitals within normal limits. Patient continues to be on room air satting 95 - 100% overnight. Net -990 mls in the past 24 hours. ECG in the past 24 hours without A-fib or flutter

02/02/2024
No acute events overnight, remains afebrile with stable vital signs satting at 98% on 2 L of oxygen overnight. Net -730 mL in the past 24 hours. Sodium down to 130 from 135, hemoglobin of 7.9 from 7.5 . ECG in the last 24 hours without A-fib or flutter. Creatinine remains normal.

2/03/2024
Patient seen at the bedside today she is still complaining of neck pain but denies any radiation or radiculopathy. She is concerned about her decreased appetite over the past 4 days. She is denying any nausea or vomiting. She is not having any palpitations or chest pain. She is still having some swelling at her ankles Lasix is on hold for her hyponatremia.
[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 135/75 (BP Location: Left arm, Patient Position: Lying) | Pulse 89 | Temp 37.2 °C (99 °F) (Oral) | Resp 18 | Ht 1.676 m (5' 6") | Wt (P) 66.4 kg (146 lb 6.2 oz) | LMP (LMP Unknown) | SpO2 94% | Breastfeeding No | BMI (P) 23.63 kg/m²
I/O: In: 1110 [P.O.:1110]
Out: 800 [Urine:800]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.** Patient has no evidence of oropharyngeal thrush
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
Ext-+2 pitting edema around ankles and bilateral feet
Skin-warm, dry, acyanotic. Peeling of bi
lateral palms, R > L-improving .
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	02/03/24
0542	02/02/24
0521
WBC	K/mcL	4.5	5.3
HGB	g/dL	**8.0***	**7.9***
MCV	fL	**101***	**101***
PLT	K/mcL	**496***	**513***
SODIUM	mmol/L	**131***	**130***
POTASSIUM	mmol/L	4.3	4.5
CHLORIDE	mmol/L	102	100
CO2	mmol/L	29	30
BUN	mg/dL	**19***	**19***
CREATININE	mg/dL	0.82	0.87
CALCIUM	mg/dL	8.6	**8.3***
GLUCOSE	mg/dL	99	88

ALBUMIN	g/dL	**3.0***	**2.8***
AST	Units/L	**42***	28
ALT	Units/L	24	19
BILIRUBIN TOTAL	mg/dL	**0.1***	**<0.1***
ALK PHOS	Units/L	76	71

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch

| Result | Value |
| | Strep Pneumo Ag,UR | Positive (A) |
| **Urine culture** |
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Bo
dy Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Status post chest tube. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # retropharyngeal and prevertebral fluid collection.
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
- Patient continues to complain of cervical neck pain but is denying any radicular symptoms
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast if symptomatic since MRI was not completely accurate from motion.
-CT cervical spine with contrast performed 01/29 showed . Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI
- IR will not be able to drain the area.
- Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-) Peak line placed 1/30

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically

improved and has HFrEF
- PRN Duonebs
-Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-)
-Pulmonary team, signed off 1/30/24, open to re-consultation if need be, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

#Peeling/desquamation of right palm-improving with current cream.
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt.
**-Given patient's low ejection fraction had a discussion with her regarding anticoagulation as well as risk benefits which could include catastrophic bleeding from any organ however she did have evidence of atrial fibrillation and is at risk of possible stroke given her CHA2DS2-VASc score of 2.
Did discuss with patient that her A-fib may have just been in the setting of acute illness but it is hard to tell if she could have paroxysmal A-fib. At this time she agrees with proceeding with anticoagulation and understands the risks. Will start her on Eliquis 5 mg twice daily.**

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg.
-Continue losartan and will transition over to Entresto in 1 to 2 days
-Strict I's and O's while on Lasix

#**Uptrending Creatinine- **normalized
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Will resume Lasix 02/01/2024. Holding Lasix currently given increased diuresis and hyponatremia
-Continue losartan

#Hypertension
-Carvedilol 3.125 mg p.o. twice daily and losartan 25 mg daily

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Start patient on Eliquis 5 mg twice daily given her A-fib with RVR but will monitor for any evidence of bleeding. Risks and benefits discussed with patient
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Suspect due to swelling at the ankle from her CHF is patient does state at times she feels it bilaterally
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Hyponatremia, likely SIADH d/t lung pathology-135 -> 130
-Fluid restriction to 1.5 L daily
-Is/Os -730 ml in the last 24 hours. Total intake of 220 mls.
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265, 01/20/2024.
- Continue to monitor serum sodium.
- Will hold lasix.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance, case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

Amanda R De Four, MD

Electronically signed by Amanda de Four, MD at 02/03/2024 9:21 PM PST

Kris S - 02/03/2024 9:42 AM PST
Formatting of this note might be different from the original.

[table#1]:
	02/03/24 0941
Oxygen Therapy/Pulse Ox
Oxygen Therapy	None (Room air)
O2 Flow Rate (L/min)	0 L/min
FiO2 (%)	21 %
SpO2	94 %
Patient Activity	At rest
Safety Instructions	Yes (Comment)
$Oxygen Initiated/Discontinued	Discontinued
$ Pulse Oximetry Spot Check	Yes
Skin Assessment
Skin Assessment	WDL
Vital Signs
Heart Rate	87
Heart
Rate Source	Monitor
Resp	18
Patient Position	Lying
Respiratory Assessment
Respiratory Pattern	Normal
Chest Assessment	Symmetrical
Cough	None
R Upper Breath Sounds	Clear
R Lower Breath Sounds	Diminished
L Upper Breath Sounds	Clear
L Lower Breath Sounds	Diminished

Supplemental oxygen discontinued. No distress noted at this time.

Electronically signed by Kris S at 02/03/2024 9:42 AM PST

Howie Que, DPM - 02/02/2024 5:18 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/02/24 0935
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/02/24 0925 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/22/24 1001
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/02/24 0925
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/02/24 1525
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Ani, Samuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/02/24 1404
• enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 02/02/24 0926
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado
, Pedro Manuel, MD, 20 mg at 02/01/24 0908
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Ani, Samuel, MD, 0.5 mg at 02/02/24 0933
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/02/24 1624
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/02/24 1632
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/02/24 0434
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/02/24 1654
• losartan (COZAAR) tablet 25 mg, 25 mg, oral, At bedtime, de Four, Amanda R, MD
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/01/24 2108
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/02/24 0159 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Ani, Samuel, MD, 500 mg at 02/02/24 0925
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/02/24 0925
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 0053
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Ani, Samuel, MD, 5 mg at 02/01/24 0355
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/01/24 0143

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	02/02/24 1630
BP:	
Pulse:	91
Resp:	18
Temp:	
SpO2:	94%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/02/24
0521
SODIUM	mmol/L	**130***
POTASSIUM	mmol/L	4.5
CHLORIDE	mm
ol/L	100
CO2	mmol/L	30
BUN	mg/dL	**19***
CREATININE	mg/dL	0.87
GLUCOSE	mg/dL	88
CALCIUM	mg/dL	**8.3***

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/02/245:19 PM

Electronically signed by Howie Que, DPM at 02/02/2024 5:20 PM PST

Hai Shao, MD, PhD - 02/02/2024 2:36 PM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
No fever overnight, feeling well with no new complaints.

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:
ammonium lactate, , topical, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
enoxaparin, 40 mg, subcutaneous, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
losartan, 25 mg, oral, At bedtime
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 01/22/24 1001
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 02/02/24 0933
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 02/01/24 2109
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/02/24 0912
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 02/01/24 2108
ondansetron, 4 mg, Q6H PRN, 4 mg at 02/02/24 0159
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
naproxen, 500 mg, BID PRN, 500 mg at 02/02/24 0053
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 02/01/24 0355
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/01/24 0143

Current Vital Signs:
[table#2]:
Vitals:
	02/02/24 0920	02/02/24 1021	02/02/24 1108	02/02/24 1238
BP:	116/72		114/79		
BP Location:	Left arm		Left arm		
Patient Position:	Sitting		Sitting		
Pulse:	91		79		
Resp:		20	20		
Temp:			36.9 °C (98.4 °F)		
TempSrc:			Oral		
SpO2:	100%	92%	93%	97%
Weight:					
Height:					

Physical Exam
Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No
tenderness. No edema.
Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	02/02/24
0521	02/01/24
0545	01/31/24
0608
WBC	K/mcL	5.3	5.2	5.6
HGB	g/dL	**7.9***	**7.5***	**8.0***
HEMATOCRIT	%	**26.2***	**24.6***	**26.1***
PLT	K/mcL	**513***	**573***	**598***
NEUTROS PCT AUTO	%	44.9	44.4	45.8
LYMPHS PCT AUTO	%	39.9	39.6	39.7
MONOS PCT AUTO	%	8.6	9.8	7.9
EOS PCT AUTO	%	4.4	4.2	5.0

[table#4]:
Results from last 7 days
Lab	Units	02/02/24
0521	02/01/24
0545	01/31/24
0610
SODIUM	mmol/L	**130***	**135***	**135***
POTASSIUM	mmol/L	4.5	4.2	4.3
CHLORIDE	mmol/L	100	102	100
CO2	mmol/L	30	**36***	**37***
BUN	mg/dL	**19***	17	**20***
CREATININE	mg/dL	0.87	0.70	0.70
CALCIUM	mg/dL	**8.3***	**8.3***	**8.3***
BILIRUBIN TOTAL	mg/dL	**<0.1***	**<0.1***	0.2
ALK PHOS	Units/L	71	71	88
ALT	Units/L	19	19	17
AST	Units/L	28	28	32
GLUCOSE	mg/dL	88	90	92

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM

	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated Final
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/
16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM

	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

IMAGING:
IR picc line placement - us/fluoro guidance as needed

Result Date: 1/30/2024
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance. release

CT cervical spine with contrast

Result Date: 1/29/2024
IMPRESSION: 1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 1/29/2024
IMPRESSION: Negative for deep vein thrombosis in both lower extremities. release

X-ray chest 2 views with pa/ap and lateral

Result Date: 1/29/2024
IMPRESSION: No significant change from prior. release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with osteophytes. CRP is better but ESR still grossly elevated. D/w RN, continue rocephin (day 8/56) and flagyl (day 8/14). Picc line placement and discharge planning.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 02/02/2024 2:47 PM PST

Sandra S - 02/02/2024 10:24 AM PST
Summary: o2 rounds

Formatting of this note might be different from the original.

[table#1]:
	02/02/24 1021
Oxygen Therapy/Pulse Ox
Oxygen Therapy	None (Room air)
O2 Delivery Method	Other (Comment)
O2 Flow Rate (L/min)	0 L/min
FiO2 (%)	21 %
SpO2	92 %
Home Oxygen	N/A
Patient Activity	At rest
$Oxygen Initiated/Discontinued	Discontinued
$ Pulse Oximetry Spot Check	Yes
Skin Assessment
Skin Assessment	WDL
Vital Signs
Resp	20

Pt weaned to room air = 92%
Charting was updated. O2 charge was d/c

Sandra M. Sandoval-Medina, RT, 2/2/2024, 10:25 AM

Electronically signed by Sandra S at 02/02/2024 10:25 AM PST

Samuel Ani, DO - 02/02/2024 7:22 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She then started receiving tPA's and dornase through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

***1/23/2024: ***Complaint of bilateral neck pain with movement, R>L. This is chronic and has been ongoing for many years, she believes it is headache vs migraine, but can clearly trace the pain towards the nape of the neck.
1/24/24: Colchicine dose decreased, slight improvement in foot pain. Medically cleared for discharge to SNF, but her insurance is not being accepted by SNFs.
1/25/24: Severe headache, mostly occipital and seems to be of cervical origin although cannot rule out other causes of headache. Fluctuating, no clear pattern or triggers. No fever, no neck rigidity. Colchicine discontinued.
1/26/24: MRI showed discitis and prevertebral fluid collection. Case discussed with ID - Dr. Shao, ENT - Dr. Hauff and IR.
**1/27/24: **ENT recommended CT w/ contrast if no improvement. Restart CHF meds and add IV lasix.
Skin peeling/desquamation over the hands only. Patient reported extensive work up in NJ for autoimmune diseases.

1/29/2024: No acute events overnight. Patient has no acute complaints today. She does not answer if she has neck pain today and is hyper focused on getting her records scanned. Will scan her prior records in our system.

1/30/2024
Patient seen and examined at bedside. No acute events overnight. Continues to be afebrile. Currently on room air satting at 97%.

Net +545 in the past 24 hours. Normal WBC. Creatinine uptrending after CT with contrast. CT cervical spine with contrast performed 01/29 showed Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. Vascular ultrasound of lower extremities performed 01/29/2024 showed no intramural thrombus. chest x-ray performed 01/29 showed Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.

01/31/24
No acute events overnight, BP of 146/93 and SpO2 of 87 noted at 0 349 today, Currently 95% on room air. Net -1 L in the past 24 hours. losartan and Lasix held due to increased creatinine 01/30/2024, and patient currently on Coreg 3.125 mg twice daily. Thoracentesis not performed by pulmonary, Dr. Jones, 01/30/2024, because of low vol of pleural effusion.

02/01/2024
Reported anxiety overnight which responded to medication. Patient downgraded to MedSurg on 01/31/2024. No new complaints and vitals within normal limits. Patient continues to be on room air satting 95 - 100% overnight. Net -990 mls in the past 24 hours. ECG in the past 24 hours without A-fib or flutter

02/02/2024
No acute events overnight, remains afebrile with stable vital signs satting at 98% on 2 L of oxygen overnight. Net -730 mL in the past 24 hours. Sodium down to 130 from 135, hemoglobin of 7.9 from 7.5 . ECG in the last 24 hours without A-fib or flutter. Creatinine remains normal.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per H
PI.

Objective

[table#3]:
Physical Exam
BP 127/71 (BP Location: Left arm, Patient Position: Lying) | Pulse 83 | Temp 36.7 °C (98.1 °F) (Oral) | Resp 19 | Ht 1.676 m (5' 6") | Wt 67.6 kg (149 lb 0.5 oz) | LMP (LMP Unknown) | SpO2 98% | Breastfeeding No | BMI 24.05 kg/m^2
I/O: In: 220 [P.O.:220]
Out: 950 [Urine:950]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
MSK- deformity of the bilateral toes; +2 pitting edema in bilateral feet. Deformity and swelling of the fingers bilaterally especially MIP.
Skin-warm, dry, acyanotic. Peeling of bilateral palms, R > L-improving .
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	02/02/24
0521	02/01/24
0545
WBC	K/mcL	5.3	5.2
HGB	g/dL	**7.9***	**7.5***
MCV	fL	**101***	100
PLT	K/mcL	**513***	**573***
SODIUM	mmol/L	**130***	**135***
POTASSIUM	mmol/L	4.5	4.2
CHLORIDE	mmol/L	100	102
CO2	mmol/L	30	**36***
BUN	mg/dL	**19***	17
CREATININE	mg/dL	0.87	0.70

CALCIUM	mg/dL	**8.3***	**8.3***
GLUCOSE	mg/dL	88	90
ALBUMIN	g/dL	**2.8***	**2.7***
AST	Units/L	28	28
ALT	Units/L	19	19
BILIRUBIN TOTAL	mg/dL	**<0.1***	**<0.1***
ALK PHOS	Units/L	71	71

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM

	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen

| **Strep pneumo antigen, urine** |
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # Eval for retropharyngeal and prevertebral abscess vs fluid collection.
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
- Patient continues to complain of cervical neck pain, no new FND although the pain was not resolving with pain management, patient has been on abx since admission and now on IV abx again.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast if symptomatic since MRI was not completely accurate from motion.
-CT cervical spine with contrast performed 01/29 showed . Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI
- IR will not be able to drain the area.
- Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-) Peak line placed 1/30

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative

-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
-
s/p Rocephin and Flagyl (1/12-1/21), transition to** **Augmentin then back to CTX/Flagyl when discitis diagnosed.
- PRN Duonebs
-Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-)
-Pulmonary team, signed off 1/30/24, open to re-consultation if need be, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

#Peeling/desquamation of right palm-improving with current cream.
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt.
-Telemetry monitoring-overight with sinus tachycardia, without A-fib or flutter.
-outpatient evaluation for anticoagulation need.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg.
-Lisinopril restarted
-Strict I's and O's while on Lasix

#**Uptrending Creatinine- **normalized
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Will resume Lasix 02/01/2024.
-Will resume Losartan 02/02/2024 if creatinine remains normal.

#Hypertension
Currently on Coreg 3.125 twice daily as losartan was held yesterday due to elevated creatinine thought to be from contrast nephropathy. Lasix held for the same reason. Blood pressure this morning 146/93 but normalized to 119/86, later in the morning, 01/311/2024.
-Will continue to monitor

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 7
-No active bleeding sites identified
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Hyponatremia, likely SIADH d/t lung pathology-135 -> 130
-Fluid restriction to 1.5 L daily
-Is/Os -730 ml in the last 24 hours. Total intake of 220 mls.
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265, 01/20/2024.

- Continue to monitor serum sodium.
- Will hold lasix for today.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance, case manag
er working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

**Resident: **
Samuel Ani, DO
Internal medicine resident

Attending: de Four, Amanda R, MD

Cosigned by Amanda de Four, MD at 02/02/2024 7:45 PM PST
Electronically signed by Samuel Ani, DO at 02/02/2024 11:05 AM PST
Electronically signed by Amanda de Four, MD at 02/02/2024 7:45 PM PST

Associated attestation - Amanda de Four, MD - 02/02/2024 7:45 PM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident on rounds. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note below as needed. I agree with and verify the findings and plan outlined except as noted below

Your Scripps Health Team - 02/01/2024 3:14 PM PST
Formatting of this note might be different from the original.
[image]

Case Management Chart Review/DCP:

**Dx/Plan: **Pt admitted for sepsis and empysema. Pt is currently pending placement for long term IV abx but underfunded.

**PLOF: **Ind, homeless. Normally lives in Huntington Beach CA
**
Funding: **Mcal

DCP: Evolving. Not a candidate for alternative bed. Email sent to admin to add to call list.

**Barrier: **underfunded

Channy RN
Inpatient Care Manager
Scripps Mercy Hospital

Electronically signed by Your Scripps Health Team at 02/01/2024 3:16 PM PST

Hai Shao, MD, PhD - 02/01/2024 3:07 PM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
No fever overnight, feeling well with no new complaints.

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:
ammonium lactate, , topical, BID

carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
enoxaparin, 40 mg, subcutaneous, Daily
furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
[Held by provider] losartan, 12.5 mg, oral, At bedtime
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 01/22/24 1001
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 02/01/24 1446
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 01/31/24 2039
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 02/01/24 0900
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 01/24/24 2104
ondansetron, 4 mg, Q6H PRN, 4 mg at 01/29/24 0343
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
naproxen, 500 mg, BID PRN, 500 mg at 01/31/24 1923
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 02/01/24 0355
zolpidem, 5 mg, At bedtime PRN, 5 mg at 02/01/24 0143

Current Vital Signs:
[table#2]:
Vitals:
	02/01/24 0850	02/01/24 0900	02/01/24 1119	02/01/24 1129
BP:	129/87		135/85	
BP Location:	Left arm		Left arm	
Patient Position:	Sitting		Lying	
Pulse:	107		95	104
Resp:	18		18	
Temp:			37.2 °C (99 °F)	
TempSrc:			Oral	
SpO2:	95%	95%	94%	94%
Weight:				
Height:				

Physical Exam
Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:

Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Musculoskeletal:
General: Normal range of motion.
Cervica
l back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No tenderness. No edema.
Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	02/01/24
0545	01/31/24
0608	01/30/24
0521
WBC	K/mcL	5.2	5.6	4.3
HGB	g/dL	**7.5***	**8.0***	**7.1***
HEMATOCRIT	%	**24.6***	**26.1***	**23.8***
PLT	K/mcL	**573***	**598***	**496***
NEUTROS PCT AUTO	%	44.4	45.8	41.3
LYMPHS PCT AUTO	%	39.6	39.7	38.3
MONOS PCT AUTO	%	9.8	7.9	11.7
EOS PCT AUTO	%	4.2	5.0	7.0

[table#4]:
Results from last 7 days
Lab	Units	02/01/24
0545	01/31/24
0610	01/30/24
0521
SODIUM	mmol/L	**135***	**135***	**135***
POTASSIUM	mmol/L	4.2	4.3	4.4
CHLORIDE	mmol/L	102	100	99
CO2	mmol/L	**36***	**37***	**37***
BUN	mg/dL	17	**20***	**27***
CREATININE	mg/dL	0.70	0.70	**1.22***
CALCIUM	mg/dL	**8.3***	**8.3***	**8.1***
BILIRUBIN TOTAL	mg/dL	**<0.1***	0.2	**0.1***
ALK PHOS	Units/L	71	88	75
ALT	Units/L	19	17	14
AST	Units/L	28	32	23
GLUCOSE	mg/dL	90	92	100

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected

| | Influenza B PCR | Not Detected |
| **COVID-19, Rapid (PCR/NAAT COMBO)** |
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: U
rine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	

| | | NO Methicillin/Oxacillin Resistant Staphylococcus aureus |
| **Wound/Fluid/Tissue Culture with stain** |
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

IMAGING:
IR picc line placement - us/fluoro guidance as needed

Result Date: 1/30/2024
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance. release

CT cervical spine with contrast

Result Date: 1/29/2024
IMPRESSION: 1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 1/29/2024
IMPRESSION: Negative for deep vein thrombosis in both lower extremities. release

X-ray chest 2 views with pa/ap and lateral

Result Date: 1/29/2024
IMPRESSION: No significant change from prior. release

MRI cervical spine with and without contrast

Result Date: 1/26/2024
Impression: Contrast imaging is very limited due to extensive motion. Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema. Findings are concerning for discitis/osteomyelitis at C4-C5. release

MRI cervical spine without contrast

Result Date: 1/25/2024
IMPRESSION: 1. Abnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation. 2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels. Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024. release 1419 A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

MRI brain without contrast

Result Date: 1/25/2024
IMPRESSION: 1. No acute intracranial findings. 2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change. 3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process. release

ASSESSMENT:
[table#6]:
| **Patient Active Problem List** |

| --- |
| Diagnosis |
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic he
art failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with osteophytes. CRP is better but ESR still grossly elevated. D/w RN, continue rocephin (day 7/56) and flagyl (day 7/14). Picc line placement and discharge planning.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 02/01/2024 3:08 PM PST

Your Scripps Health Team - 02/01/2024 9:58 AM PST
Formatting of this note is different from the original.
Clinical Nutrition Monitor Note

Recommendations to Provider:
Continue Regular diet. 1500 mL fluid restriction per provider.
Continue multivitamin with minerals daily.

Nutrition Diagnosis (PES statement):
Inadequate oral Intake related to previous mechanical ventilation, now extubated with PO diet as evidenced by average PO intake greater than 75% of meals, now meeting estimated kcal needs. -RESOLVED.

Dietary Orders
(From admission, onward)

Start Ordered
01/22/24 1101 Combination Diet Regular; 1500 mL; Dental Soft Diet effective now
Question Answer Comment
Please Select Base Diet: Regular
Fluid Restriction - Oral (mL/day): 1500 mL
Food Consistency: Dental Soft

01/22/24 1100

Food Allergies: Lactose Intolerance

Nutritionally significant labs:
Lab Results
Component Value Date/Time
NA 135 (L) 02/01/2024 05:45 AM

Nutritionally significant medications:
Bowel Regimen
furosemide (LASIX) tablet 20 mg
multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet

Skin: intact

Anthropometrics:
Height: 167.6 cm (5' 6"), Weight: 71.7 kg (158 lb), BMI (Calculated): 25.5

Weight trend this admission: [] Loss [] Gain [x] Stable [] Requested new weight
Weight change assessment: N/A

Estimated needs:
Weight Used for Equation Calculations: 63.6 kg (140 lb 3.4 oz), Describe Weight Used: Actual
Total Daily Calorie Estimated Needs: 1600-1900 (25-30 kcal/kg) kcal/day
Total Daily Protein Estimated Needs : 65-75 (1-1.2 g/kg) g/day
Total Daily Fluid Estimated Needs: 1600-1900 mL/day

Nutrition Related Summary since last visit:
Patient working with other clinicians at time of visit. Chart reviewed.
Regular, Dental soft diet ordered.
Average PO intake 89% x 9 meals.
Average intake based on selections: 5025 kcal and 170 gm protein daily.
Patient is exceeding 100% of nutrition needs.
Weight trend stable.
Nutrition labs stable.
Last BM date 1/30; WNL.
Closing care plan d/t 1 week of demonstrated adequate intake with good GI tolerance and stable weight trend.

Is patient meeting estimating needs?: [x] Yes [] No
Are current nutrition related orders adequate and appropriate?: [x] Yes [] No
Change recommended: N/A

Plan of Care Discussion: IDT Rounds

Nutrition Level of Care: Moderate (LC2) --> Low (LC1)

Sarah P, RD
Clinical Dietitian
RD Office: 312-7176

Electronically signed by Your Scripps Health Team at 02/01/2024 12:53 PM PST

Howie Que, DPM - 02/01/2024 8:00 AM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use

alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 02/02/24 0935
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 02/02/24 0925 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/22/24 1001
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 02/02/24 0925
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 02/02/24 1525
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Ani, Samuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 02/02/24 1404
• enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 02/02/24 0926
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 02/01/24 0908
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Ani, Samuel, MD, 0.5 mg at 02/02/24 0933
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 02/02/24 1624
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 02/02/24 1632
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/02/24 0434
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 02/02/24 1654
• losartan (COZAAR) tablet 25 mg, 25 mg, oral, At bedtime, de Four, Amanda R, MD
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 02/01/24 2108
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 02/02/24 0159 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Ani, Samuel, MD, 500 mg at 02/02/24 0925
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 02/02/24 0925
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 02/02/24 0053
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Ani, Samuel, MD, 5 mg at 02/01/24 0355
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 02/01/24 0143

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	02/02/24 1630
BP:	
Pulse:	91
Resp:	18
Temp:	
SpO2:	94%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral

to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	02/02/24
0521
SODIUM	mmol/L	**130***
POTASSIUM	mmol/L	4.5
CHLORIDE	mmol/L	100
CO2	mmol/L	30
BUN	mg/dL	**19***
CREATININE	mg/dL	0.87
GLUCOSE	mg/dL	88
CALCIUM	mg/dL	**8.3***

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular respons
e (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
02/02/245:20 PM

Electronically signed by Howie Que, DPM at 02/02/2024 5:21 PM PST

Samuel Ani, DO - 02/01/2024 6:46 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She then started receiving tPA's and dornase through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle

pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

***1/23/2024: ***Complaint of bilateral neck pain with movement, R>L. This is chronic and has been ongoing for many years, she believes it is headache vs migraine, but can clearly trace the pain towards the nape of the neck.
1/24/24: Colchicine dose decreased, slight improvement in foot pain. Medically cleared for discharge to SNF, but her insurance is not being accepted by SNFs.
1/25/24: Severe headache, mostly occipital and seems to be of cervical origin although cannot rule out other causes of headache. Fluctuating, no clear pattern or triggers. No fever, no neck rigidity. Colchicine discontinued.
1/26/24: MRI showed discitis and prevertebral fluid collection. Case discussed with ID - Dr. Shao, ENT - Dr. Hauff and IR.
**1/27/24: **ENT recommended CT w/ contrast if no improvement. Restart CHF meds and add IV lasix.
Skin peeling/desquamation over the hands only. Patient reported extensive work up in NJ for autoimmune diseases.

1/29/2024: No acute events overnight. Patient has no acute complaints today. She does not answer if she has neck pain today and is hyper focused on getting her records scanned. Will scan her prior records in our system.

1/30/2024
Patient seen and examined at bedside. No acute events overnight. Continues to be afebrile. Currently on room air satting at 97%. Net +545 in the past 24 hours. Normal WBC. Creatinine uptrending after CT with contrast. CT cervical spine with contrast performed 01/29 showed Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. Vascular ultrasound of lower extremities performed 01/29/2024 showed no intramural thrombus. chest x-ray performed 01/29 showed Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.

01/31/24
No acute events overnight, BP of 146/93 and SpO2 of 87 noted at 0 349 today, Currently 95% on room air. Net -1 L in the past 24 hours. losartan and Lasix held due to increased creatinine 01/30/2024, and patient currently on Coreg 3.125 mg twice daily. Thoracentesis not performed by pulmonary, Dr. Jones, 01/30/2024, because of low vol of pleural effusion.

02/01/2024
Reported anxiety overnight which re
sponded to medication. Patient downgraded to MedSurg on 01/31/2024. No new complaints and vitals within normal limits. Patient continues to be on room air satting 95 - 100% overnight. Net -990 mls in the past 24 hours. ECG in the past 24 hours without A-fib or flutter

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 145/89 (BP Location: Left arm, Patient Position: Lying) | Pulse 97 | Temp 36.8 °C (98.2 °F) (Oral) | Resp 20 | Ht 1.676 m (5' 6") | Wt 71.7 kg (158 lb) | LMP (LMP Unknown) | SpO2 93% | Breastfeeding No | BMI 25.50 kg/m²
I/O: In: 1020 [P.O.:1020]
Out: 1500 [Urine:1500]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
MSK- deformity of the bilateral toes; +2 pitting edema in bilateral feet. Deformity and swelling of the fingers bilaterally especially MIP.
Skin-warm, dry, acyanotic. Peeling of bilateral palms, R > L-improving .
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	02/01/24

0545 | 01/31/24
0610 | 01/31/24
0608 | 01/30/24
0521 |
WBC	K/mcL	5.2	--	5.6	4.3
HGB	g/dL	**7.5***	--	**8.0***	**7.1***
MCV	fL	100	--	**102***	**102***
PLT	K/mcL	**573***	--	**598***	**496***
SODIUM	mmol/L	--	**135***	--	**135***
POTASSIUM	mmol/L	--	4.3	--	4.4
CHLORIDE	mmol/L	--	100	--	99
CO2	mmol/L	--	**37***	--	**37***
BUN	mg/dL	--	**20***	--	**27***
CREATININE	mg/dL	--	0.70	--	**1.22***
CALCIUM	mg/dL	--	**8.3***	--	**8.1***
GLUCOSE	mg/dL	--	92	--	100
ALBUMIN	g/dL	--	**3.0***	--	**2.6***
AST	Units/L	--	32	--	23
ALT	Units/L	--	17	--	14
BILIRUBIN TOTAL	mg/dL	--	0.2	--	**0.1***
ALK PHOS	Units/L	--	88	--	75

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collectio
n Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date

| **Wound/Fluid/Tissue Culture with stain** |
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

No results found.

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # Eval for retropharyngeal and prevertebral abscess vs fluid collection.
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
- Patient continues to complain of cervical neck pain, no new FND although the pain was not resolving with pain management, patient has been on abx since admission and now on IV abx again.

- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast if symptomatic since MRI was not completely accurate from motion.
-CT cervical spine with contrast performed 01/29 showed . Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI
- IR will not be able to drain the area.
- Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-\
) Peak line placed 1/30
- Dr. De Four will seek the opinion of neurosurgery regarding the paravertebral fluid collection.

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- s/p Rocephin and Flagyl (1/12-1/21), transition to** **Augmentin then back to CTX/Flagyl when discitis diagnosed.
- PRN Duonebs
-Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-)
-Pulmonary, Dr. Jones, planned thoracentesis 1/30/202-cancelled for low volume, recommends restarting Advair close to discharge and possible nocturnal O2 study prior to discharge-as patient continues to desat at night.
-Pulmonary team, signed off 1/30/24, open to re-consultation if need be, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

#Peeling/desquamation of right palm-improving with current cream.
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt.
-Telemetry monitoring-overight with sinus tachycardia, without A-fib or flutter.
-outpatient evaluation for anticoagulation need.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg.
-Lisinopril restarted
-Strict I's and O's while on Lasix

#**Uptrending Creatinine- **normalized
 # Possible contrast-induced AKI -resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024 and 0.7 on 02/01/2024.
-Will resume Lasix 02/01/2024.
-Will resume Losartan 02/02/2024 if creatinine remains normal.

#Hypertension
Currently on Coreg 3.125 twice daily as losartan was held yesterday due to elevated creatinine thought to be from contrast nephropathy. Lasix held for the same reason. Blood pressure this morning 146/93 but normalized to 119/86, later in the morning, 01/311/2024.
-Will continue to monitor

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
- folate-normal at 10.5, B12-normal at 718, obtained on 01/31/2024.
-will transfuse if < 6.
-No active bleeding sites identified
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024

- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefi
t with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Hyponatremia, likely SIADH d/t lung pathology-improving
Fluid restriction to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265, 01/20/2024.
- Continue to monitor serum sodium.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance, case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

**Resident: **
Samuel Ani, DO
Internal medicine resident

Attending: de Four, Amanda R, MD

Cosigned by Amanda de Four, MD at 02/01/2024 9:48 PM PST
Electronically signed by Samuel Ani, DO at 02/01/2024 10:43 AM PST
Electronically signed by Amanda de Four, MD at 02/01/2024 9:48 PM PST

Associated attestation - Amanda de Four, MD - 02/01/2024 9:48 PM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident on rounds. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note below as needed. I agree with and verify the findings and plan outlined except as noted below

Howie Que, DPM - 01/31/2024 7:29 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History

Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Ani, Samuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 01/31/24 0846
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Ani, Samuel, MD, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/31/24 0846 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Ani, Samuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/22/24 1001
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Ani, Samuel, MD, 3.125 mg at 01/31/24 0846
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g,
2 g, intraVENOUS, Q24H, Ani, Samuel, MD, 2 g at 01/31/24 1617
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Ani, Samuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Ani, Samuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Ani, Samuel, MD, 15 mL at 01/31/24 1741
• enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/31/24 0846
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Ani, Samuel, MD, 0.5 mg at 01/31/24 0300
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Ani, Samuel, MD, 25 mg at 01/29/24 0343
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Ani, Samuel, MD, 3 mL at 01/31/24 1648
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Ani, Samuel, MD, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 01/31/24 0416
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Ani, Samuel, MD, 1 patch at 01/31/24 1618
• [Held by provider] losartan (COZAAR) tablet 12.5 mg, 12.5 mg, oral, At bedtime, Umukoro, Emuejevuoke Anastasia, DO
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Ani, Samuel, MD, 10 mg at 01/24/24 2104
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/29/24 0343 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Ani, Samuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Ani, Samuel, MD, 500 mg at 01/31/24 0846
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Ani, Samuel, MD, 1 tablet at 01/31/24 0846
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/31/24 1923
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Ani, Samuel, MD, 5 mg at 01/26/24 1834
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/30/24 2214

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	01/31/24 1656
BP:	
Pulse:	98
Resp:	18
Temp:	
SpO2:	100%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very
pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.

[table#2]:
Results from last 7 days
Lab	Units	01/31/24
0610
SODIUM	mmol/L	**135***
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BUN	mg/dL	**20***
CREATININE	mg/dL	0.70
GLUCOSE	mg/dL	92
CALCIUM	mg/dL	**8.3***

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
01/31/247:29 PM

Electronically signed by Howie Que, DPM at 01/31/2024 7:29 PM PST

Hai Shao, MD, PhD - 01/31/2024 12:38 PM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
No fever overnight, feeling well with no new complaints.

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:

ammonium lactate, , topical, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
enoxaparin, 40 mg, subcutaneous, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
[Held by provider] losartan, 12.5 mg, oral, At bedtime
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 01/22/24 1001
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 01/31/24 0300
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 01/29/24 0343
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 01/31/24 1102
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 01/24/24 2104
ondansetron, 4 mg, Q6H PRN, 4 mg at 01/29/24 0343
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
naproxen, 500 mg, BID PRN, 500 mg at 01/31/24 0630
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 01/26/24 1834
zolpidem, 5 mg, At bedtime PRN, 5 mg at 01/30/24 2214

Current Vital Signs:
[table#2]:
Vitals:
	01/31/24 0800	01/31/24 1102	01/31/24 1110	01/31/24 1134
BP:				**(!) 129/94**
BP Location:				Left arm
Patient Position:				Sitting
Pulse:	111	101		97
Resp:		20	20	20
Temp:				36.9 °C (98.4 °F)
TempSrc:				Oral
SpO2:		94%	100%	96%
Weight:				
Height:				

Physical Exam
Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumati
c.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No tenderness. No edema.
Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	01/31/24
0608	01/30/24
0521	01/29/24
0543
WBC	K/mcL	5.6	4.3	4.6
HGB	g/dL	**8.0***	**7.1***	**7.7***
HEMATOCRIT	%	**26.1***	**23.8***	**25.2***
PLT	K/mcL	**598***	**496***	**552***
NEUTROS PCT AUTO	%	45.8	41.3	53.3
LYMPHS PCT AUTO	%	39.7	38.3	27.5
MONOS PCT AUTO	%	7.9	11.7	10.2
EOS PCT AUTO	%	5.0	7.0	7.1

[table#4]:
Results from last 7 days
Lab	Units	01/31/24
0610	01/30/24
0521	01/29/24
0543
SODIUM	mmol/L	**135***	**135***	**131***
POTASSIUM	mmol/L	4.3	4.4	4.5
CHLORIDE	mmol/L	100	99	**97***
CO2	mmol/L	**37***	**37***	**36***
BUN	mg/dL	**20***	**27***	**20***
CREATININE	mg/dL	0.70	**1.22***	0.75
CALCIUM	mg/dL	**8.3***	**8.1***	**8.2***
BILIRUBIN TOTAL	mg/dL	0.2	**0.1***	0.2
ALK PHOS	Units/L	88	75	86
ALT	Units/L	17	14	16
AST	Units/L	32	23	25
GLUCOSE	mg/dL	92	100	101

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value

| | Influenza A PCR | Not Detected |
| | Influenza B PCR | Not Detected |
| **COVID-19, Rapid (PCR/NAAT COMBO)** |
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM

	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value

| | Staph Screen culture | |
| | | NO Methicillin/Oxacillin Resistant Staphylococcus aureus |
| **Wound/Fluid/Tissue Culture with stain** |
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final

IMAGING:
IR picc line placement - us/fluoro guidance as needed

Result Date: 1/30/2024
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance. release

CT cervical spine with contrast

Result Date: 1/29/2024
IMPRESSION: 1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 1/29/2024
IMPRESSION: Negative for deep vein thrombosis in both lower extremities. release

X-ray chest 2 views with pa/ap and lateral

Result Date: 1/29/2024
IMPRESSION: No significant change from prior. release

MRI cervical spine with and without contrast

Result Date: 1/26/2024
Impression: Contrast imaging is very limited due to extensive motion. Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema. Findings are concerning for discitis/osteomyelitis at C4-C5. release

MRI cervical spine without contrast

Result Date: 1/25/2024
IMPRESSION: 1. Abnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation. 2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels. Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024. release 1419 A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Fi
nding System upon receiving acknowledgement.

MRI brain without contrast

Result Date: 1/25/2024
IMPRESSION: 1. No acute intracranial findings. 2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change. 3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process. release

X-ray chest single frontal view

Result Date: 1/25/2024
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with osteophytes. CRP is better but ESR still grossly elevated. D/w RN, continue rocephin (day 6/56) and flagyl (day 6/14). Picc line placement and discharge planning.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 01/31/2024 12:39 PM PST

Samuel Ani, DO - 01/31/2024 6:55 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She then started receiving tPA's and dornase through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

***1/23/2024: ***Complaint of bilateral neck pain with movement, R>L. This is chronic and has been ongoing for many years, she believes it is headache vs migraine, but can clearly trace the pain towards the nape of the neck.
1/24/24: Colchicine dose decreased, slight improvement in foot pain. Medically cleared for discharge to SNF, but her insurance is not being accepted by SNFs.
1/25/24: Severe headache, mostly occipital and seems to be of cervical origin although cannot rule out other causes of headache. Fluctuating, no clear pattern or triggers. No fever, no neck rigidity. Colchicine discontinued.
1/26/24: MRI showed discitis and prevertebral fluid collection. Case discussed with ID - Dr. Shao, ENT - Dr. Hauff and IR.
**1/27/24: **ENT recommended CT w/ contrast if no improvement. Restart CHF meds and add IV lasix.
Skin peeling/desquamation over the hands only. Patient reported extensive work up in NJ for autoimmune diseases.

1/29/2024: No acute events overnight. Patient has no acute complaints today. She does not answer if she has neck pain today and is hyper focused on getting her records scanned. Will scan her prior records in our system.

1/30/2024
Patient seen and examined at bedside. No acute events overnight. Continues to be afebrile. Currently on room air satting at 97%. Net +545 in the past 24 hours. Normal WBC. Creatinine uptrending after CT with contrast. CT cervical spine with contrast

performed 01/29 showed Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. Vascular ultrasound of lower extremities performed 01/29/2024 showed no intramural thrombus. chest x-ray performed 01/29 showed Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.

01/31/24
No acute events overnight, BP of 146/93 and SpO2 of 87 noted at 0 349 today, Currently 95% on room air. Net -1 L in the past 24 hours. losartan and Lasix held due to increased creatinine 01/30/2024, and patient currently on Coreg 3.125 mg twice daily. Thoracentesis not performed by pulmonary, Dr. Jones, 01/30/2024, because of low vol of pleural effusion.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 119/86 (BP Location: Left arm, Patient Position: Sitting) | Pulse 111 | Temp 36.9 °C (98.4 °F) (Oral) | Resp 20 | Ht 1.676 m (5' 6") | Wt 69.8 kg (153 lb 14.1 oz) | LMP (LMP Unknown) | SpO2 94% | Breastfeeding No | BMI 24.84 kg/m²
I/O: In: 840 [P.O.:840]
Out: 1850 [Urine:1850]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
MSK- deformity of the bilateral toes; +2 pitting edema in bilateral feet.
Skin-warm, dry, acyanotic. Peeling of bilateral palms, R > L
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/31/24
0610	01/31/24
0608	01/30/24
0521
WBC	K/mcL	--	5.6	4.3
HGB	g/dL	--	**8.0***	**7.1***
MCV	fL	--	**102***	**102***
PLT	K/mcL	--	**598***	**496***
SODIUM	mmol/L	**135***	--	**135***
POTASSIUM	mmol/L	4.3	--	4.4
CHLORIDE	mmol/L	100	--	99
CO2	mmol/L	**37***	--	**37***
BUN	mg/dL	**20***	--	**27***
CREATININE	mg/dL	0.70	--	**1.22***
CALCIUM	mg/dL	**8.3***	--	**8.1***
GLUCOSE	mg/dL	92	--	100
ALBUMIN	g/dL	**3.0***	--	**2.6***
AST	Units/L	32	--	23
ALT	Units/L	17	--	14
BILIRUBIN TOTAL	mg/dL	0.2	--	**0.1***
ALK PHOS	Units/L	88	--	75

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B

	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAA
T)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance

	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth Final
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date

IR picc line placement - us/fluoro guidance as needed

Result Date: 1/30/2024
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance. release

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
**# Eval for retropharynge
al and prevertebral abscess vs fluid collection.**
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
- Patient continues to complain of cervical neck pain, no new FND although the pain was not resolving with pain management, patient has been on abx since admission and now on IV abx again.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast if symptomatic since MRI was not completely accurate from motion.
-CT cervical spine with contrast performed 01/29 showed . Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI
- IR will not be able to drain the area.
- Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-) Peak line placed 1/30
- Dr. De Four will seek the opinion of neurosurgery regarding the paravertebral fluid collection.

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- s/p Rocephin and Flagyl (1/12-1/21), transition to** **Augmentin then back to CTX/Flagyl when discitis diagnosed.
- PRN Duonebs
-Rocephin for 56 days (1/25-) and Flagyl for 14 days: (1/25-)

-Pulmonary, Dr. Jones, planned thoracentesis 1/30/202-cancelled for low volume, recommends restarting Advair close to discharge and possible nocturnal O2 study prior to discharge-as patient continues to desat at night.
-Pulmonary team, signed off 1/30/24, open to re-consultation if need be, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

#Peeling/desquamation of right palm-improving with current cream.
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt.
-Telemetry monitoring-recently with sinus tachycardia, without A-fib or flutter.
-outpatient evaluation for anticoagulation need.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg.
-Lisinopril restarted
-Strict I's and O's while on Lasix

#**Uptrending creatinine- **normalized
 # Possible contrast-induced AKI-resolved.
-Creatinine noted to be 1.22 01/30 from 0.75 on 1/29/2024; 0.7 1/31/2024
-Hold Lasix, will resume, starting with 20 mg 02/02/2024, if creatinine remains normal
-Hold losartan and monitor blood pressure

#Hypertension
Currently on Coreg 3.125 twice daily as losartan was held yesterday due to elevated creatinine thought to be from contrast nephropathy. Lasix held for the same reason. Blood pressure this morning 146/93 but normalized to 119/86, later in the morning, 01/311/2024.
-Will continue to monitor

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
-Check folate, B12, and iron studies.
-will transfus
e if < 6.
-No active bleeding sites identified
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Hyponatremia, likely SIADH d/t lung pathology-improving
Fluid restriction to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265, 01/20/2024.
- Continue to monitor serum sodium.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.

-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with current insurance, case manager working with patient to change insurance to provide SNF coverage and hinted will take weeks to change.

**Resident: **
Samuel Ani, DO
Internal medicine resident

Attending: de Four, Amanda R, MD

Cosigned by Amanda de Four, MD at 01/31/2024 10:57 PM PST
Electronically signed by Samuel Ani, DO at 01/31/2024 11:22 AM PST
Electronically signed by Amanda de Four, MD at 01/31/2024 10:57 PM PST

Associated attestation - Amanda de Four, MD - 01/31/2024 10:57 PM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident on rounds. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note below as needed. I agree with and verify the findings and plan outlined except as noted below

Howie Que, DPM - 01/30/2024 4:57 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen \
(TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• ammonium lactate (LAC-HYDRIN) 12 % lotion, , topical, BID, Ani, Samuel, MD, Given at 01/30/24 1510
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/30/24 0830 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Chen, Yu-Chun, DO, 500 mg at 01/22/24 1001
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Umukoro, Emuejevuoke Anastasia, DO, 3.125 mg at 01/30/24 0829
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Shao, Hai, MD, PhD, 2 g at 01/30/24 1510
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD

• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Dammert Coronado, Pedro Manuel, MD, 15 mL at 01/30/24 1252
• enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/29/24 0847
• [Held by provider] furosemide (LASIX) injection 40 mg, 40 mg, intraVENOUS, Daily, Umukoro, Emuejevuoke Anastasia, DO
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• heparin PF 1000 unit/NS 500 mL BAG, , , Continuous PRN, Nelson, Trevor D, MD, 500 mL at 01/30/24 1548
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD, 0.5 mg at 01/29/24 2355
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Chen, Yu-Chun, DO, 25 mg at 01/29/24 0343
• ipratropium-albuterol (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/30/24 1034
• lidocaine (XYLOCAINE) 20 mg/mL (2 %) vial, , , PRN, Nelson, Trevor D, MD, 5 mL at 01/30/24 1547
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/30/24 0353
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Piatetsky, Yana, MD, 1 patch at 01/30/24 1635
• [START ON 1/31/2024] losartan (COZAAR) tablet 12.5 mg, 12.5 mg, oral, At bedtime, Umukoro, Emuejevuoke Anastasia, DO
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/24/24 2104
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/29/24 0343 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Shao, Hai, MD, PhD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/30/24 0829
• naproxen (NAPROSYN
) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/30/24 0618
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/26/24 1834
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/28/24 2101

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	01/30/24 1643
BP:	123/83
Pulse:	88
Resp:	18
Temp:	37 °C (98.6 °F)
SpO2:	97%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.

[table#2]:
Results from last 7 days
Lab	Units	01/30/24
0521
SODIUM	mmol/L	**135***
POTASSIUM	mmol/L	4.4
CHLORIDE	mmol/L	99
CO2	mmol/L	**37***
BUN	mg/dL	**27***
CREATININE	mg/dL	**1.22***
GLUCOSE	mg/dL	100
CALCIUM	mg/dL	**8.1***

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
01/30/244:57 PM

Electronically signed by Howie Que, DPM at 01/30/2024 4:58 PM PST

Hai Shao, MD, PhD - 01/30/2024 2:35 PM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
No fever overnight, feeling well with mild neck pain in the back. Some migraine headache. "I had diskitis a few years ago at New Jersey."

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:
ammonium lactate, , topical, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
enoxaparin, 40 mg, subcutaneous, Daily
[Held by provider] furosemide, 40 mg, intraVENOUS, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
[START ON 1/31/2024] losartan, 12.5 mg, oral, At bedtime
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

**Current PRN Medica

tions:**
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 01/22/24 1001
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 01/29/24 2355
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 01/29/24 0343
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 01/30/24 1034
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 01/24/24 2104
ondansetron, 4 mg, Q6H PRN, 4 mg at 01/29/24 0343
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
naproxen, 500 mg, BID PRN, 500 mg at 01/30/24 0618
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 01/26/24 1834
zolpidem, 5 mg, At bedtime PRN, 5 mg at 01/28/24 2101

Current Vital Signs:
[table#2]:
Vitals:
	01/30/24 0510	01/30/24 0707	01/30/24 0709	01/30/24 1103
BP:		103/83		115/79
BP Location:		Left arm		Left arm
Patient Position:		Sitting		Lying
Pulse:		101	104	91
Resp:		17	18	17
Temp:		36.9 °C (98.4 °F)		37 °C (98.6 °F)
TempSrc:		Oral		Oral
SpO2:		**(!) 89%**	91%	93%
Weight:	66.1 kg (145 lb 11.6 oz)			
Height:				

Physical Exam
Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.
Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds. No decreased breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no hepatomegaly or mass.
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Left lower leg: No tenderness. No edema.

Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Findings: No ecchymosis.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	01/30/24
0521	01/29/24
0543	01/27/24
0553	01/25/24
0531	01/24/24
0548
WBC	K/mcL	4.3	4.6	4.8	< >	4.8
HGB	g/dL	**7.1***	**7.7***	**7.6***	< >	**8.0***
HEMATOCRIT	%	**23.8***	**25.2***	**24.3***	< >	**26.3***
PLT	K/mcL	**496***	**552***	**452***	< >	364
NEUTROS PCT AUTO	%	41.3	53.3	--	--	65.7
LYMPHS PCT AUTO	%	38.3	27.5	--	--	19.3
MONOS PCT AUTO	%	11.7	10.2	--	--	11.1
EOS PCT AUTO	%	7.0	7.1	--	--	2.7
< > = values in this interval not displayed.

[table#4]:
Results from last 7 days
Lab	Units	01/30/24
0521	01/29/24
0543	01/28/24
0540
SODIUM	mmol/L	**135***	**131***	**130***
POTASSIUM	mmol/L
4.4	4.5	4.3
CHLORIDE	mmol/L	99	**97***	**94***
CO2	mmol/L	**37***	**36***	**36***
BUN	mg/dL	**27***	**20***	14
CREATININE	mg/dL	**1.22***	0.75	0.67
CALCIUM	mg/dL	**8.1***	**8.2***	8.4
BILIRUBIN TOTAL	mg/dL	**0.1***	0.2	0.2
ALK PHOS	Units/L	75	86	81
ALT	Units/L	14	16	16
AST	Units/L	23	25	32
GLUCOSE	mg/dL	100	101	97

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch

Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM

	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth to Date
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date

IMAGING:
CT cervical sp
ine with contrast

Result Date: 1/29/2024
IMPRESSION: 1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 1/29/2024
IMPRESSION: Negative for deep vein thrombosis in both lower extremities. release

X-ray chest 2 views with pa/ap and lateral

Result Date: 1/29/2024
IMPRESSION: No significant change from prior. release

MRI cervical spine with and without contrast

Result Date: 1/26/2024
Impression: Contrast imaging is very limited due to extensive motion. Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema. Findings are concerning for discitis/osteomyelitis at C4-C5. release

MRI cervical spine without contrast

Result Date: 1/25/2024
IMPRESSION: 1. Abnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation. 2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels. Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024. release 1419 A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

MRI brain without contrast

Result Date: 1/25/2024
IMPRESSION: 1. No acute intracranial findings. 2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change. 3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process. release

X-ray chest single frontal view

Result Date: 1/25/2024
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion

•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. Reviewed pt's printed medical records, she had MRI C-spine in 2/2019 that showed only degenerative disease with osteophytes. CRP is better but ESR still grossly elevated. D/w RN, continue rocephin (day 5/56) and flagyl (day 5/14). Picc line placement and discharge planning.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 01/30/2024 2:51 PM PST

Christopher Jones, MD - 01/30/2024 1:01 PM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista
History of Present Illness:
This is hospital day 18 for Elizabeth Tina Petruccio a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

Copied from progress note 1/27

Interval Events:
Intubated, chest tube placed, extubated after short time, improvement with chest tube and antibiotics. Chest tube removed 1/20. Thoracentesis 1/25 completed for persistent loculated fluid

1/25 MRI notable for cervical C4-C5 edema wi
th paravertebral collection extending from C2-C7, concern for osteomyelitis discitis without a definitive epidural collection

Copied from progress note 1/27

24 Hour Events:
No acute events overnight, patient remains compliant with her I-S and has been achieving around 1700 cc per her reports. Awake alert, no complaints today, says that her various aches and pains seem to be on the up and up.

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Intravenous Lines/Tubes/Drains:
All lines and tubes were reviewed on 1/30/2024 and continued to be medically required.
[table#2]:
Patient Devices

[table#3]:
| | [table#3.1]
[table#3.2]
[table#3.3] |
| --- | --- |

[table#3.1]:
Active Devices

[table#3.2]:
	Name	Placement date	Placement time	Site	Days
	Midline Single Lumen 01/14/24 Right Basilic	01/14/24	0215	Basilic	16
	External Urinary Device	—	—	—	17

[table#3.3]:

[table#4]:

Medications:
Scheduled Meds:ammonium lactate, , topical, BID
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
[Held by provider] enoxaparin, 40 mg, subcutaneous, Daily
[Held by provider] furosemide, 40 mg, intraVENOUS, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
[START ON 1/31/2024] losartan, 12.5 mg, oral, At bedtime
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate, oral, TID PRN, Dao, Diep Ngoc, DO
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate, oral, BID PRN, Chen, Yu-Chun, DO
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• hydrOXYzine pamoate, oral, TID PRN, Chen, Yu-Chun, DO
• ipratropium-albuteroL, nebulization, Q4H PRN, Pena, Jose Rafael, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Diet:
[table#5]:
|
|
| --- |
| [table#5.1] |

[table#5.1]:
Dietary Orders
(From admission, onward)

[table#6]:
|
|
| --- |

[table#7]:
| | [table#7.1]
[table#7.2] |
| --- | --- |

[table#7.1]:
	Start			Ordered
	01/22/24 1101		[table#7.1.1]	01/22/24 1100

[table#7.1.1]:
Combination Diet Regular; 1500 mL; Dental Soft Diet effective now
[table#7.1.1.1]

[table#7.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Fluid Restriction - Oral (mL/day):	1500 mL	
Food Consistency:	Dental Soft	

[table#7.2]:

[table#8]:

Allergies on file: Lactose intolerant [lactose]

Review of Systems:
12 organ review of systems was negative other than HPI

Physical Exam:
[table#9]:
Vitals:
	01/30/24 1103
BP:	115/79
Pulse:	91
Resp:	17
Temp:	37 °C (98.6 °F)
SpO2:	93%

[table#10]:
| Temp: [36.6 °C (97.9 °F)-37 °C (98.6 °F)] 37 °C (98.6 °F)
Heart Rate: [88-104] 91
Resp: [16-24] 17
BP: (103-128)/(72-83) 115/79
FiO2 (%): [21 %] 21 % | SpO2: 93 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 0 L/min | Height: 167.6 cm (5' 6")
Weight: 66.1 kg (145 lb 11.6 oz)
BMI (Calculated): 23.5 |
| --- | --- | --- |
Cardiac Rhythm: Sinus tachycardia
In: 1345 [P.O.:1330; I.V.:15]
Out: 800 [Urine:800]

Intake/Output Summary (Last 24 hours) at 1/30/2024 1303
Last data filed at 1/30/2024 0707
[table#11]:
Gross per 24 hour
Intake	1000 ml
Output	600 ml
Net	**400 ml**

[table#12]:
Last 5 weights
	01/13/24 0205	01/22/24 0411	01/25/24 1900	01/29/24 0409
Weight:	63.6 kg (140 lb 3.4 oz)	65.3 kg (144 lb)	68 kg (149 lb 14.6 oz)	66 kg (145 lb 8.1 oz)
	01/30/24 0510
Weight:	66.1 kg (145 lb 11.6 oz)

FiO2 (%): [21 %] 21 %

EXAM:
GEN: awake, no apparent distress on room air in good spirits
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist membranes, clear nares
EYES: PER, anicteric sclera
HEART: RRR, no murmur
LUNGS: clear to auscultation, no wheezing
ABD: soft, NT, ND, + BS
EXT/MSK: no LE edema, 2+ bil LE pulses
NEURO/PSYCH: awake and alert, moving all 4 extremities spontaneously
SKIN: no rashes, no skin lesions

Laboratory Data:

[table#13]:
Results from last 7 days
Lab	Units	01/30/24
0521	01/29/24
0543	01/27/24
0553
WBC	K/mcL	4.3	4.6	4.8
HGB	g/dL	**7.1***	**7.7***	**7.6***
HEMATOCRIT	%	**23.8***	**25.2***	**24.3***
PLT	K/mcL	**496***	**552***	**452***

[table#14]:
Results from last 7 days
Lab	Units	01/30/24
0521	01/29/24
0543	01/28/24
0540
SODIUM	mmol/L	**135***	**131***	**130***
POTASSIUM	mmol/L	4.4	4.5	4.3
CHLORIDE	mmol/L	99	**97***	**94***
CO2	mmol/L	**37***	**36***	**36***
BUN	mg/dL	**27***	**20***	14
CREATININE	mg/dL	**1.22***	0.75	0.67
CALCIUM	mg/dL	**8.1***	**8.2***	8.4
ALBUMIN	g/dL	**2.6***	**2.8***	**2.9***
BILIRUBIN TOTAL	mg/dL	**0.1***	0.2	0.2
ALK PHOS	Units/L	75	86	81
ALT	Units/L	14	16	16
AST	Units/L	23	25	32
GLUCOSE	mg/dL	100	101	97

[table#15]:
Results from last 7 days
Lab	Units	01/25/24
0531
LD	Units/L	141

No lab exists for component: "CS30M"

[table#16]:
Lab Results

Component	Value	Date/Time
	BLOODCULT	No Growth to Date	01/26/2024 10:32 AM
	BLOODCULT	No Growth to Date	01/26/2024 10:23 AM

Urinalysis:
[table#17]:
POCT
Component	Value	Date
	PHU	6.0	01/16/2024
	SPECGRAVU	1.025	01/16/2024
	COLORU	Yellow	01/16/2024
	CLARITYU	Clear	01/16/2024
	KETONESU	Negative	01/16/2024
	WBCU	0-2	01/16/2024
	RBCU	0-2	01/16/2024
	PROTEINUR	30 (A)	01/16/2024
	UROBILINOGEN	0.2	01/16/2024
	NITRITEU	Positive (A)
01/16/2024 |

Cultures:

[table#18]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date

| **Wound/Fluid/Tissue Culture with stain** |
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth to Date
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date

Studies:
Reviewed CXR PA and lat 1/29 showing anterior fluid based collection R lung, clear left lung, no pneumothorax

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure now resolved , likely secondary to strep pneumo empyema
2. COPD
3. Right sided empyema, status post chest tube and thoracentesis for loculated fluid collection, on antibiotics
4. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes on acyclovir
5. Heart failure, reduc
ed EF
6. Discitis and osteomyelitis of C4-C6, possible high risk for respiratory depression give lesion location
7. Anemia

**Plan: **

1. Agree with ceftriaxone and Flagyl, prolonged course per ID 8 weeks seem reasonable and will also cover her Pulmonary infection.
2. Follow-up blood cultures are no growth to date 1/29
3. Continue to monitor incentive spirometry, discussed with patient today continues to chart this

4. Will monitor respiratory muscle strength with incentive spirometer, set goal as 1250 cc which patient was able to achieve with deep breathing. If patient has persistent decline in her respiratory function greater than 500 cc, may need to consider and transfer to ICU for airway watch given cervical spine location of phlegmon
5. - s/p ultrasound guided- thoracentesis of right lung base, 300cc of pleural fluid exudative in nature, pH 7.272 will likely not require repeat chest tube.
6. See procedure note from today, no tappable fluid, chronic fibrous material(likely old blood and inflammation) remains in the right pleural cavity, notable on chest x-ray. This should continue to resolve on its own as her body heals
7. EF was low before in 2020, 25 to 30%. Likely not worth repeating echo at this time,
8. Cont coreg, losartan, lasix prn per hospitalist, Lasix on hold for AKI 1/30
9. **Restart Advair closer to discharge**, patient has been requiring routine inhalers . Patient reports having PFTs in the past but I am unable to find any evidence of this in Care Everywhere despite having access to records from other states, reviewed records with her at bedside 1/29 without pfts noted.
10. Continue to monitor nocturnal oxygen saturation has intermittently required low amounts of oxygen overnight** may need nocturnal O2 study prior to discharge (unclear when that will be)**

#Prophylaxis:
DVT prophylaxis: Resume Lovenox as above

#CODE STATUS full code

DVT screen: (-) 1/29
Follow-up will sign off at this time, please reconsult Pulmonary if there is been any change in her status. Recommend to continue incentive spirometry monitoring but would have expected there to have been progression of her respiratory insufficiency by this point. Patient appears to be doing well on current antibiotic regiment

Discussed plan of care with primary team.

Disposition:
Per primary team.

Christopher Jones, MD
#802462
Pulmonary & Critical Care Medicine

Total time evaluating patient, speaking with patient/family, reviewing records, laboratory data, creating, coordinating and communicating treatment plan, excluding procedures: 39 minutes

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a moderate complexity case affecting the respiratory system.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Christopher Jones, MD at 01/30/2024 1:07 PM PST

Samuel Ani, DO - 01/30/2024 6:53 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Li
kely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She then started receiving tPA's and dornase through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

***1/23/2024: ***Complaint of bilateral neck pain with movement, R>L. This is chronic and has been ongoing for many years, she believes it is headache vs migraine, but can clearly trace the pain towards the nape of the neck.
1/24/24: Colchicine dose decreased, slight improvement in foot pain. Medically cleared for discharge to SNF, but her insurance is not being accepted by SNFs.
1/25/24: Severe headache, mostly occipital and seems to be of cervical origin although cannot rule out other causes of headache. Fluctuating, no clear pattern or triggers. No fever, no neck rigidity. Colchicine discontinued.
1/26/24: MRI showed discitis and prevertebral fluid collection. Case discussed with ID - Dr. Shao, ENT - Dr. Hauff and IR.
**1/27/24: **ENT recommended CT w/ contrast if no improvement. Restart CHF meds and add IV lasix.
Skin peeling/desquamation over the hands only. Patient reported extensive work up in NJ for autoimmune diseases.

1/29/2024: No acute events overnight. Patient has no acute complaints today. She does not answer if she has neck pain today and is hyper focused on getting her records scanned. Will scan her prior records in our system.

1/30/2024
Patient seen and examined at bedside. No acute events overnight. Continues to be afebrile. Currently on room air satting at 97%. Net +545 in the past 24 hours. Normal WBC. Creatinine uptrending after CT with contrast. CT cervical spine with contrast performed 01/29 showed Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. Vascular ultrasound of lower extremities performed 01/29/2024 showed no intramural thrombus. chest x-ray performed 01/29 showed Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 103/83 (BP Location: Left arm, Patient Position: Sitting) | Pulse 104 | Temp 36.9 °C (98.4 °F) (Oral) | Resp 18 | Ht 1.676 m (5' 6") | Wt 66.1 kg (145 lb 11.6 oz) | LMP (LMP Unknown) | SpO2 91% | Breastfeeding No | BMI 23.52 kg/m²
I/O: In: 1345 [P.O.:1330; I.V.:15]
Out: 800 [Urine:800]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
MSK- deformity of the bilateral toes; +2 pitting edema in bilateral feet.
Skin-warm, dry, acyanotic. Peeling of bilateral palms, R > L
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no
focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/30/24
0521	01/29/24
0543
WBC	K/mcL	4.3	4.6
HGB	g/dL	**7.1***	**7.7***
MCV	fL	**102***	**102***
PLT	K/mcL	**496***	**552***

SODIUM	mmol/L	**135***	**131***
POTASSIUM	mmol/L	4.4	4.5
CHLORIDE	mmol/L	99	**97***
CO2	mmol/L	**37***	**36***
BUN	mg/dL	**27***	**20***
CREATININE	mg/dL	**1.22***	0.75
CALCIUM	mg/dL	**8.1***	**8.2***
GLUCOSE	mg/dL	100	101
ALBUMIN	g/dL	**2.6***	**2.8***
AST	Units/L	23	25
ALT	Units/L	14	16
BILIRUBIN TOTAL	mg/dL	**0.1***	0.2
ALK PHOS	Units/L	75	86

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM

	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No
organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth to Date
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date

CT cervical spine with contrast

Result Date: 1/29/2024
IMPRESSION: 1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI. release

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 1/29/2024
IMPRESSION: Negative for deep vein thrombosis in both lower extremities. release

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024. Recent complaint of neck pain s/p CT neck with contrast revealed features consistent with cervical discitis-osteomyelitis and patient currently on an 8-week course of Rocephin and flagyl, started 1/25.

 # Cervical discitis/osteomyelitis C4-C6
 # Eval for retropharyngeal and prevertebral abscess vs fluid collection.
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
- Patient continues to complain of cervical neck pain, no new FND although the pain was not resolving with pain management,

patient has been on abx since admission and now on IV abx again.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast if symptomatic since MRI was not completely accurate from motion.
-CT cervical spine with contrast performed 01/29 showed . Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis. 2. Persistent prevertebral fluid, probably similar in extent to the recent MRI
- IR will not be able to drain the area.
- Rocephin for 56 days (1/5-) and Flagyl for 14 days: (1/5-)
- Peak line placement ordered 1/30

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
-Patient currently on room air but desats at night needing supplemental oxygen.
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- s/p Rocephin and Flagyl (1/12-1/21), transition to** **Augmentin then back to CTX/Flagyl when discitis diagnosed.
- PRN Duonebs
-Rocephin for 56 days (1/5-) and Flagyl for 14 days: (1/5-)
-Pulmonary, Dr. Jones, plans thoracentesis 1/30/202, recommends restarting Advair close to discharge and possible nocturnal O2 study prior to discharge-as patient continues to desat at night.
-Pulmonary team, signing off 1/30/24, open to re-consultation if need be, as the remaining pleural effusion is likely to resolve spontaneously, per Dr. Jones.

**#Peeling/desquamation of r
ight palm**
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care- Lac hydrin cream and monitor.

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt.
-Telemetry monitoring-recently with sinus tachycardia, without A-fib or flutter.
-outpatient evaluation for anticoagulation need.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg.
-Lisinopril restarted
-Strict I's and O's while on Lasix

#**Uptrending creatinine**
 # Possible contrast-induced AKI
-Creatinine noted to be 1.22 this morning from 0.75 on 1/29/2024
-Given that patient received contrast for neck CT, 1/29/2024 , she is likely having contrast induced AKI
-Will hold nephrotoxic medications,
-Hold Lasix.
-Hold losartan and monitor blood pressure

**# Macrocytic anemia **
-Hemoglobin of 7.1 1/30 down-trending from 7.7 on 01/29, with MCV of 102
-Check folate, B12, and iron studies.
-will transfuse if < 6.
-No active bleeding sites identified
-Continue to monitor

**# Right ankle/foot pain, likely gout **-improving
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 4 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Arthrocentesis yielded insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- S/p IV acyclovir 400 mg
- S/p oral clotrimazole

 # Hyponatremia, likely SIADH d/t lung pathology-improving
Fluid restriction to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265, 01/20/2024.
- Continue to monitor serum sodium.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with insurance.

**Resident: **
Samuel Ani, DO
Internal medicine resident

Attending: de Four, Amanda R, MD

Cosigned by Amanda de Four, MD at 01/30/2024 9:29 PM PST
Electronically signed by Samuel Ani, DO at 01/30/2024 1:11 PM PST
Electronically signed by Samuel Ani, DO at 01/30/2024 1:28 PM PST
Electronically signed by Samuel Ani, DO at 01/30/2024 1:40 PM PST
Electronically signed by Samuel Ani, DO at 01/30/2024 3:08 PM PST
Electronically signed by Amanda de Four, MD at 01/30/2024 9:29 PM PST

Associated attestation - Amanda de Four, MD - 01/30/2024 9:29 PM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident on rounds. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note below as needed. I agree with and verify the findings and plan outlined except as noted below

Howie Que, DPM - 01/29/2024 7:55 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness an
d has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/29/24 0847 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Chen, Yu-Chun, DO, 500 mg at 01/22/24 1001
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Umukoro, Emuejevuoke Anastasia, DO, 3.125 mg at 01/29/24 0847
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Khayyat, Omar Salim Peter, MD, 2 g at 01/28/24 2052
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Dammert Coronado, Pedro Manuel, MD, 15 mL at 01/29/24 1821
• [Held by provider] enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/29/24 0847
• [START ON 1/30/2024] furosemide (LASIX) injection 40 mg, 40 mg, intraVENOUS, Daily, Umukoro, Emuejevuoke Anastasia, DO
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD, 0.5 mg at 01/29/24 0050
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Chen, Yu-Chun, DO, 25 mg at 01/29/24 0343
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/29/24 1852
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/29/24 0347
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Piatetsky, Yana, MD, 1 patch at 01/29/24 1645
• [START ON 1/30/
2024] lisinopriL (PRINIVIL) tablet 10 mg, 10 mg, oral, Daily, Umukoro, Emuejevuoke Anastasia, DO
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/24/24 2104
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/29/24 0343 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Shao, Hai, MD, PhD, 500 mg at 01/29/24 0847
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/29/24 0847
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/28/24 2101
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/26/24 1834
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/28/24 2101

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	01/29/24 1855
BP:	
Pulse:	
Resp:	
Temp:	

| SpO2: | 93% |

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.

[table#2]:
Results from last 7 days
Lab	Units	01/29/24
0543
SODIUM	mmol/L	**131***
POTASSIUM	mmol/L	4.5
CHLORIDE	mmol/L	**97***
CO2	mmol/L	**36***
BUN	mg/dL	**20***
CREATININE	mg/dL	0.75
GLUCOSE	mg/dL	101
CALCIUM	mg/dL	**8.2***

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
01/29/247:55 PM

Electronically signed by Howie Que, DPM at 01/29/2024 7:56 PM PST

Christopher Jones, MD - 01/29/2024 7:05 PM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista
History of Present Illness:
This is hospital day 17 for Elizabeth Tina Petruccio a 59 y.o. female BIBM from border patrol CC: Sh
ortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

Copied from progress note 1/27

Interval Events:
Intubated, chest tube placed, extubated after short time, improvement with chest tube and antibiotics. Chest tube removed 1/20. Thoracentesis 1/25 completed for persistent loculated fluid

1/25 MRI notable for cervical C4-C5 edema with paravertebral collection extending from C2-C7, concern for osteomyelitis discitis without a definitive epidural collection

Copied from progress note 1/27

24 Hour Events:
No acute events over the weekend.
Continues to chart IS, for the most part maintinaing >1250 cc
Naproxen helping with headache, denies breathing changes.

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Intravenous Lines/Tubes/Drains:
All lines and tubes were reviewed on 1/29/2024 and continued to be medically required.
[table#2]:
Patient Devices

[table#3]:
| | [table#3.1]
[table#3.2]
[table#3.3] |
| --- | --- |

[table#3.1]:
Active Devices

[table#3.2]:
	Name	Placement date	Placement time	Site	Days
	Midline Single Lumen 01/14/24 Right Basilic	01/14/24	0215	Basilic	15
	External Urinary Device	—	—	—	16

[table#3.3]:

[table#4]:

Medications:
Scheduled Meds:carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
[Held by provider] enoxaparin, 40 mg, subcutaneous, Daily
[START ON 1/30/2024] furosemide, 40 mg, intraVENOUS, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
[START ON 1/30/2024] lisinopriL, 10 mg, oral, Daily
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate, oral, TID PRN, Dao, Diep Ngoc, DO
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate, oral, BID PRN, Chen, Yu-Chun, DO
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• hydrOXYzine pamoate, oral, TID PRN, Chen, Yu-Chun, DO
• ipratropium-albuteroL, nebulization, Q4H PRN, Pena, Jose Rafael, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Diet:
[table#5]:
|
|
| --- |
| [table#5.1] |

[table#5.1]:
Dietary Orders
(From admission, onward)

[table#6]:
|
|
| --- |

[table#7]:
|
| [table#7.1]
[table#7.2] |
| --- | --- |

[table#7.1]:
	Start			Ordered
	01/22/24 1101		[table#7.1.1]	01/22/24 1100

[table#7.1.1]:
Combination Diet Regular; 1500 mL; Dental Soft Diet effective now
[table#7.1.1.1]

[table#7.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Fluid Restriction - Oral (mL/day):	1500 mL	
Food Consistency:	Dental Soft	

[table#7.2]:

[table#8]:

Allergies on file: Lactose intolerant [lactose]

Review of Systems:
12 organ review of systems was negative other than HPI
Physical Exam:
[table#9]:
Vitals:
	01/29/24 1855
BP:	
Pulse:	
Resp:	
Temp:	
SpO2:	93%

[table#10]:
| Temp: [36.5 °C (97.7 °F)-37.1 °C (98.8 °F)] 36.7 °C (98.1 °F)
Heart Rate: [80-107] 98
Resp: [16-23] 23
BP: (91-111)/(62-72) 110/72 | SpO2: 93 %
O2 Flow Rate (L/min): 0 L/min | Height: 167.6 cm (5' 6")
Weight: 66 kg (145 lb 8.1 oz)
BMI (Calculated): 23.5 |
| --- | --- | --- |
Cardiac Rhythm: Sinus tachycardia
In: 1065 [P.O.:1050; I.V.:15]
Out: 1600 [Urine:1600]

Intake/Output Summary (Last 24 hours) at 1/29/2024 1905
Last data filed at 1/29/2024 1805
[table#11]:
Gross per 24 hour
Intake	1065 ml
Output	1600 ml
Net	**-535 ml**

[table#12]:
Last 5 weights
	01/12/24 1931	01/13/24 0205	01/22/24 0411	01/25/24 1900
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)	65.3 kg (144 lb)	68 kg (149 lb 14.6 oz)
	01/29/24 0409
Weight:	66 kg (145 lb 8.1 oz)

EXAM:
GEN: awake, no apparent distress on room air
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist membranes, clear nares
EYES: PER, anicteric sclera
HEART: RRR, no murmur
LUNGS: clear to auscultation, no wheezing
ABD: soft, NT, ND, + BS
EXT/MSK: no LE edema, 2+ bil LE pulses
NEURO/PSYCH: awake and alert, moving all 4 extremities spontaneously
SKIN: no rashes, no skin lesions

Laboratory Data:

[table#13]:
Results from last 7 days
Lab	Units	01/29/24
0543	01/27/24
0553	01/26/24
0536
WBC	K/mcL	4.6	4.8	5.7
HGB	g/dL	**7.7***	**7.6***	**8.2***
HEMATOCRIT	%	**25.2***	**24.3***	**26.7***

| PLT | K/mcL | **552*** | **452*** | **478*** |

[table#14]:
Results from last 7 days
Lab	Units	01/29/24
0543	01/28/24
0540	01/27/24
0553
SODIUM	mmol/L	**131***	**130***	**129***
POTASSIUM	mmol/L	4.5	4.3	4.4
CHLORIDE	mmol/L	**97***	**94***	**97***
CO2	mmol/L	**36***	**36***	**32***
BUN	mg/dL	**20***	14	14
CREATININE	mg/dL	0.75	0.67	0.63
CALCIUM	mg/dL	**8.2***	8.4	**8.3***
ALBUMIN	g/dL	**2.8***	**2.9***	**2.7***
BILIRUBIN TOTAL	mg/dL	0.2	0.2	0.2
ALK PHOS	Units/L	86	81	71
ALT	Units/L	16	16	16
AST	Units/L	25	32	26
GLUCOSE	mg/dL	101	97	99

[table#15]:
Results from last 7 days
Lab	Units	01/25/24
0531
LD	Units/L	141

No lab exists for component: "CS30M"

[table#16]:
Lab Results
Component	Value	Date/Time
	BLOODCULT	No Growth to Date	01/26/2024 10:32 AM
	BLOODCULT	No Growth to Date	01/26/2024 10:23 AM

Urinalysis:
[table#17]:
POCT
Component	Value	Date
	PHU	6.0	01/16/2024
	SPECGRAVU	1.025	01/16/2024
	COLORU	Yellow	01/16/2024
	CLARITYU	Clear	01/16/2024
	KETONESU	Negative	01/16/2024
	WBCU	0-2	01/16/2024
RBCU	0-2	01/16/2024
	PROTEINUR	30 (A)	01/16/2024
	UROBILINOGEN	0.2	01/16/2024
	NITRITEU	Positive (A)	01/16/2024

Cultures:

[table#18]:
Recent Results (from the past 720 hour(s))

| **Influenza A/B** |
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance

	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth to Date
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date

Studies:
F/u CXR PA and lat 1/29 showing anterior fluid based collection R lung, clear left lung, no pneumothorax

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure now resolved , likely secondary to strep pneumo empyema
2. COPD
3. Right sided empyema, status post chest tube and thoracentesis for loculated
fluid collection, on antibiotics
4. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes on acyclovir
5. Heart failure, reduced EF
6. Discitis and osteomyelitis of C4-C6, possible high risk for respiratory depression give lesion location
7. Anemia

**Plan: **

1. Agree with ceftriaxone and Flagyl, prolonged course per ID 8 weeks seem reasonable and will also cover her Pulmonary infection.
2. Follow-up blood cultures are no growth to date 1/29
3. Continue to monitor incentive spirometry, discussed with patient today continues to chart this
4. Will monitor respiratory muscle strength with incentive spirometer, set goal as 1250 cc which patient was able to achieve with deep breathing. If patient has persistent decline in her respiratory function greater than 500 cc, may need to consider and transfer to ICU for airway watch given cervical spine location of phlegmon
5. - s/p ultrasound guided- thoracentesis of right lung base, 300cc of pleural fluid exudative in nature, pH 7.272 will likely not require repeat chest tube.
1. Hold lovenox, plan for thora 1/30.
6. EF was low before in 2020, 25 to 30%. Likely not worth repeating echo at this time,
1. Cont coreg, losartan, lasix prn per hospitalist
7. Restart Advair closer to discharge, patient has been requiring routine inhalers . Patient reports having PFTs in the past but I am unable to find any evidence of this in Care Everywhere despite having access to records from other states, reviewed records with her at bedside 1/29 without pfts noted.
8. Continue to monitor nocturnal oxygen saturation has intermittently required low amounts of oxygen overnight** may need nocturnal O2 study prior to discharge (unclear when that will be)**

#Prophylaxis:
DVT prophylaxis: on hold

#CODE STATUS Full code

DVT screen: (-) 1/29
Follow-up will follow

Discussed plan of care with primary team.

Disposition:
Per primary team.

Christopher Jones, MD
#802462
Pulmonary & Critical Care Medicine

Total time evaluating patient, speaking with patient/family, reviewing records, laboratory data, creating, coordinating and communicating treatment plan, excluding procedures: 35 minutes

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a moderate complexity case affecting the respiratory system.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Christopher Jones, MD at 01/29/2024 7:12 PM PST
Electronically signed by Christopher Jones, MD at 01/29/2024 7:12 PM PST

Your Scripps Health Team - 01/29/2024 4:44 PM PST
Formatting of this note might be different from the original.
Discussed case w/ CM Supervisor, Steve, and since pt does not have a solid DCP after SNF, she will not fit qualifications for an Alternative Bed.

Barriers: Needs PT/OT and IV abx but pt has straight Mcal in process of switching from OC to SD, thus no accepting SNF at this time.

DCP evolving

Electronically signed by Your Scripps Health Team at 01/29/2024 4:45 PM PST

Emuejevuoke Umukoro, DO - 01/29/2024 2:42 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respirato
ry failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She then started receiving tPA's and dornase through her chest tube for remaining loculations. Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

***1/23/2024: ***Complaint of bilateral neck pain with movement, R>L. This is chronic and has been ongoing for many years, she believes it is headache vs migraine, but can clearly trace the pain towards the nape of the neck.
1/24/24: Colchicine dose decreased, slight improvement in foot pain. Medically cleared for discharge to SNF, but her insurance is not being accepted by SNFs.
1/25/24: Severe headache, mostly occipital and seems to be of cervical origin although cannot rule out other causes of headache. Fluctuating, no clear pattern or triggers. No fever, no neck rigidity. Colchicine discontinued.
1/26/24: MRI showed discitis and prevertebral fluid collection. Case discussed with ID - Dr. Shao, ENT - Dr. Hauff and IR.
**1/27/24: **ENT recommended CT w/ contrast if no improvement. Restart CHF meds and add IV lasix.
Skin peeling/desquamation over the hands only. Patient reported extensive work up in NJ for autoimmune diseases.

1/29/2024: No acute events overnight. Patient has no acute complaints today. She does not answer if she has neck pain today and is hyper focused on getting her records scanned. Will scan her prior records in our system.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 91/71 (BP Location: Left arm, Patient Position: Sitting) | Pulse 85 | Temp 36.9 °C (98.4 °F) (Oral) | Resp 16 | Ht 1.676 m (5' 6") | Wt 66 kg (145 lb 8.1 oz) | LMP (LMP Unknown) | SpO2 96% | Breastfeeding No | BMI 23.48 kg/m²
I/O: In: 905 [P.O.:900; I.V.:5]
Out: 800 [Urine:800]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
MSK- deformity of the bilateral toes; +2 pitting edema in bilateral feet.
Skin-warm, dry, acyanotic. Peeling of bilateral palms, R > L
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/29/24
0543	01/28/24
0540	01/27/24
0553
WBC	K/mcL	4.6	--	4.8
HGB	g/dL	**7.7***	--	**7.6***
MCV	fL	**102***	--	100
PLT	K/mcL	**552***	--	**452***
SODIUM	mmol/L	**131***	**130***	**129***
POTASSIUM	mmol/L	4.5	4.3	4.4
CHLORIDE	mmol/L	**97***	**94***	**97***
CO2	mmol/L	**36***	**36***	**32***
BUN	mg/dL	**20***	14	14
CREATININE	mg/dL	0.75	0.67	0.63
CALCIUM	mg/dL	**8.2***	8.4	**8.3***
GLUCOSE	mg/dL	101	97	99
ALBUMIN	g/dL	**2.8***	**2.9***	**2.7***
AST	Units/L	25	32	26
ALT	Units/L	16	16	16
BILIRUBIN TOTAL	mg/dL	0.2	0.2	0.2
ALK PHOS	Units/L	86	81	71

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab

| Result | Value |
| | COVID-19, Rapid (PCR/NAAT) | Not Detected |
| **Urine culture** |
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value

	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth to Date
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date

X-ray chest 2 views with pa/ap and lateral

Result Date: 1/29/2024
IMPRESSION: No significant change from prior. release

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024.

 # Cervical discitis/osteomyelitis C4-C6
 # Eval for retropharyngeal and prevertebral abscess vs fluid collection.
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
- Patient continues to complain of cervical neck pain, no new FND although the pain was not resolving with pain management, patient has been on abx since admission and now on IV abx again.
- ID consulted - Dr. Shao --> recommended high-dose CTX (for at least 8 weeks) and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast if symptomatic since MRI was not completely accurate from motion.
-Follow-up CT cervical spine results.
- Discussed case w/ IR and they will not be able to drain the area.
- CTX/Flagyl (1/25 -). Anticipate end date of 03/21/2024.

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
- Wean supplemental oxygen as tolerated
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- s/p Rocephin and Flagyl (1/12-1/21), transition to** **Augmentin then back to CTX/Flagyl when discitis diagnosed.
- PRN Duonebs

#Peeling/desquamation of right palm
Patient complained of right forearm feeling, without erythema, rash or edema. She reported prior similar episode on the left palm 1 year ago. She reported extensive autoimmune workup in New Jersey.
-Autoimmune workup: RF and CCP negative, ANA pending
-Supportive care and monitor.

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt. Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic.
-Follow up regarding anticoagulation outpatient.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg. Will restart lisinopril if tolerated.
-IV Lasix to keep patient euvolemic. Resume oral Lasix tomorrow.

-Strict I's and O's while on Lasix

 # Right ankle/foot pain, likely gout vs disseminated infection.
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Tapped the joint with insufficient aspirate for culture. Recommended continuation of colchicine treatment .
-Colchicine DC'd 1/25/2024.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis. Patient also reported benefit with lidocaine spray. Lesions have shown much improvement. HIV negative.
- s/p IV acyclovir 400 mg
- s/p oral clotrimazole

 # AKI - resolved
-Creatinine 1.2 OA, from baseline of 0.9.
- Conti
nue to avoid nephrotoxic medications

**# Hyponatremia, likely SIADH d/t lung pathology-worsening **
Fluid restriction to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265.
- Continue to monitor serum sodium.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT, recommended discharge to SNF, but no current SNF coverage with insurance.

**Resident: **Emuejevuoke Anastasia Umukoro, DO

Attending: Khayyat, Omar Salim Peter, MD

Cosigned by Omar Khayyat, MD at 01/30/2024 12:08 AM PST
Electronically signed by Emuejevuoke Umukoro, DO at 01/29/2024 3:26 PM PST
Electronically signed by Omar Khayyat, MD at 01/30/2024 12:08 AM PST

Associated attestation - Omar Khayyat, MD - 01/30/2024 12:08 AM PST
Formatting of this note might be different from the original.
I have personally examined the patient, reviewed and interpreted the studies and data, discussed the findings and plan of care in detail with the Residents Team, I agree with current plan.

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024.

Since then she had another thoracentesis and pulmonology is following.

Also she has been complaining of headache all the time while she is here and she mentioned that she has migraine but her pain was in the neck actually and when MRI done it showed C4-C5 osteo and prevertebral/retropharyngeal fluid collection, ENT and ID consulted and patient is on IV abx now and has no symptoms, her inflammatory markers are still high and repeat CT cervical spine with contrast shows no changes in the fluid collections seen on MRI, will need to touch base with Dr. Hauff (or ENT on call) for follow up.

Patient also had A fib with RVR, her CHADSVASC is 2 (female and CHF) and might possibly qualify for eliquis although female sex has been recently questioned when counting CHADSVASC, she has been getting serial procedures and still on lovenox, once no

further procedures we can start AC or or defer for outpatient since she has low CHADSVASC.

Getting GDMT for CHF but BP is slightly soft.

E&M Coding:
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the multidisciplinary team. I have reviewed available medical records, follow up on consult notes, flow sheets, medication record, orders, imaging results, and serial sequential labs.

More than 50 minutes spent in the preparation, coordination, formulation of treatment plan with resident's team, obtaining information personally, reviewing records, consult notes and discuss updates with patient, family and care management team.

MEDICAL DECISION MAKING: This is a high complexity case affecting multiple system. Patient's risk is high given multiple underlying other comorbidities as described above.

I am covering this patient from 8 AM to 10 PM. Everyday between the hours of 10 PM and 8 AM the night hospitalist will be covering this patient and can be reached at 619-600-0746.

Hai Shao, MD, PhD - 01/29/2024 2:36 PM PST
Formatting of this note might be different from the original.
Progress Note

Subjective:
Weekend events reviewed. No fever overnight, feeling well with mild neck pain in the back.

Current Antibiotics
[table#1]:
No current outpatient medications on file.

Current Scheduled Medications:
carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
[Held by provider] enoxaparin, 40 mg, subcutaneous, Daily
[START ON 1/30/2024] furosemide, 40 mg, intraVENOUS, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
losartan, 12.5 mg, oral, Daily
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
benzonatate, 100 mg, TID PRN, 100 mg at 01/26/24 1853
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
calcium carbonate, 500 mg, BID PRN, 500 mg at 01/22/24 1001
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 01/29/24 0050
hydrOXYzine pamoate, 25 mg, TID PRN, 25 mg at 01/29/24 0343
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 01/29/24 1133
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/23/24 2024
melatonin, 10 mg, At bedtime PRN, 10 mg at 01/24/24 2104
ondansetron, 4 mg, Q6H PRN, 4 mg at 01/29/24 0343
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
naproxen, 500 mg, BID PRN, 500 mg at 01/28/24 2101
oxyCODONE, 5 mg, Q4H PRN, 5 mg at 01/26/24 1834

zolpidem, 5 mg, At bedtime PRN, 5 mg at 01/28/24 2101

Current Vital Signs:
[table#2]:
Vitals:
	01/29/24 0409	01/29/24 0733	01/29/24 1133	01/29/24 1147
BP:		97/64		91/71
BP Location:		Left arm		Left arm
Patient Position:		Sitting		Sitting
Pulse:		107		85
Resp:		22		16
Temp:		36.5 °C (97.7 °F)		36.9 °C (98.4 °F)
TempSrc:		Oral		Oral
SpO2:		93%	94%	96%
Weight:	66 kg (145 lb 8.1 oz)			
Height:				

Physical Exam

LABORATORY
[table#3]:
Results from last 7 days
Lab	Units	01/29/24
0543	01/27/24
0553	01/26/24
0536	01/25/24
0531	01/24/24
0548	01/23/24
0544
WBC	K/mcL	4.6	4.8	5.7	< >	4.8	4.2
HGB	g/dL	**7.7***	**7.6***	**8.2***	< >	**8.0***	**7.8***
HEMATOCRIT	%	**25.2***	**24.3***	**26.7***	< >	**26.3***	**25.6***
PLT	K/mcL	**552***	**452***	**478***	< >	364	353
NEUTROS PCT AUTO	%	53.3	--	--	--	65.7	5
7.0
LYMPHS PCT AUTO	%	27.5	--	--	--	19.3	25.2
MONOS PCT AUTO	%	10.2	--	--	--	11.1	**14.2***
EOS PCT AUTO	%	7.1	--	--	--	2.7	2.2
< > = values in this interval not displayed.

[table#4]:
Results from last 7 days
Lab	Units	01/29/24
0543	01/28/24
0540	01/27/24
0553
SODIUM	mmol/L	**131***	**130***	**129***
POTASSIUM	mmol/L	4.5	4.3	4.4
CHLORIDE	mmol/L	**97***	**94***	**97***
CO2	mmol/L	**36***	**36***	**32***
BUN	mg/dL	**20***	14	14
CREATININE	mg/dL	0.75	0.67	0.63
CALCIUM	mg/dL	**8.2***	8.4	**8.3***
BILIRUBIN TOTAL	mg/dL	0.2	0.2	0.2
ALK PHOS	Units/L	86	81	71
ALT	Units/L	16	16	16
AST	Units/L	25	32	26
GLUCOSE	mg/dL	101	97	99

MICROBIOLOGY
[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value

| | Influenza A PCR | Not Detected |
| | Influenza B PCR | Not Detected |
| **COVID-19, Rapid (PCR/NAAT COMBO)** |
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	

| | | NO Methicillin/Oxacillin Resistant Staphylococcus aureus |
| **Wound/Fluid/Tissue Culture with stain** |
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs

| | Gram Stain | No organisms seen |
| **Blood Culture, Peripheral #1** |
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth to Date
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date

IMAGING:
X-ray chest 2 views with pa/ap and lateral

Result Date: 1/29/2024
IMPRESSION: No significant change from prior. release

MRI cervical spine with and without contrast

Result Date: 1/26/2024
Impression: Contrast imaging is very limited due to extensive motion. Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema. Findings are concerning for discitis/osteomyelitis at C4-C5. release

MRI cervical spine without contrast

Result Date: 1/25/2024
IMPRESSION: 1. Abnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation. 2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels. Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024. release 1419 A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

MRI brain without contrast

Result Date: 1/25/2024
IMPRESSION: 1. No acute intracranial findings. 2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change. 3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process. release

X-ray chest single frontal view

Result Date: 1/25/2024
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated release

ASSESSMENT:
[table#6]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)

| • | Empyema of left pleural space (CMS/HCC) |
| • | Acute osteomyelitis of cervical spine (CMS/HCC) |

PLAN:
No fever overnight with no leukocytosis, clinically stable. Sepsis workup negative. ENT recommends conservative medical treatment. D/w RN, continue rocephin and flagyl for now, check ESR and CRP.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 01/29/2024 2:52 PM PST

Emuejevuoke Umukoro, DO - 01/28/2024 4:30 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presented with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.
She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics. She then started receiving tPA's and dornase through her chest tube for remaining loculations.

Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

1/23/2023
Patient is complaining of neck pain with movement, mostly over the R side but both are painful. This is chronic and has been ongoing for many years, she believes it is headache vs migraine although the patient was able to clearly trace the pain towards the mm of the neck.

1/24
Patient's foot pain is slightly improving, colchicine dose decreased.
Patient is medically ready to be dced to SNF, unfortunately her insurance is not being accepted by SNFs.

1/25
Patient is having severe headache, mostly occipital and seems to be of cervical origin although cannot rule out other causes of headache. Seems to be fluctuating and no clear pattern or triggers for her headache. No fever, no neck rigidity.
Patient's foot pain has been improving, will discontinue colchicine tomorrow if she has no further pain.

1/26
Patient's MRI showing discitis and prevertebral fluid collection, patient hs no headache when awake, she is aware of all the updates, csae discussed with ID - Dr. Shao, ENT - Dr. Hauff and IR.
Will start GDMT as well for HFrEF.

1/27
Patient's Na+ is 129 now, patient with no new headache although still has pain when sleeping.
ENT recommended CT w/ contrast if no improvement. Restart CHF meds, she has LE edema and will add IV lasix.
She also wanted to discuss skin peeling/desquamation over the hands only.
Patient reported extensive work up in NJ for autoimmune diseases and will pull her records from her app.

1/28: Patient still reports neck pain/headache when she lays down, peeling palms R>L and lower extremity swelling. She does report prior similar peeling of her left palm a year ago. No other acute complaints.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 97/68 (BP Location: Left arm, Patient Position: Sitting) | Pulse 87 | Temp 36.9 °C (98.4 °F) (Oral) | Resp 15 | Ht 1.676 m (5' 6") | Wt 68 kg (149 lb 14.6 oz) | LMP (LMP Unknown) | SpO2 97% | Breastfeeding No | BMI 24.20 kg/m²
I/O: In: 485 [P.O.:480; I.V.:5]
Out: 653 [Urine:653]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side.
**GI - **soft, nondistended, nontender
MSK **-Mild swelling and tenderness of the right foot and deformity of the bilateral toes.**
Skin-warm, dry, acyanotic. Peeling of bilateral palms, R > L
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/28/24
0540	01/27/24
0553	01/26/24
0536
WBC	K/mcL	--	4.8	5.7
HGB	g/dL	--	**7.6***	**8.2***
MCV	fL	--	100	100
PLT	K/mcL	--	**452***	**478***
SODIUM	mmol/L
130*	**129***	**131***
POTASSIUM	mmol/L	4.3	4.4	4.5
CHLORIDE	mmol/L	**94***	**97***	98
CO2	mmol/L	**36***	**32***	**32***
BUN	mg/dL	14	14	16
CREATININE	mg/dL	0.67	0.63	0.67
CALCIUM	mg/dL	8.4	**8.3***	8.4
GLUCOSE	mg/dL	97	99	100
ALBUMIN	g/dL	**2.9***	**2.7***	**2.8***
AST	Units/L	32	26	26
ALT	Units/L	16	16	14
BILIRUBIN TOTAL	mg/dL	0.2	0.2	0.3
ALK PHOS	Units/L	81	71	89

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value

| | Urine Culture | |
| | | Multiple organisms present, probable skin/urogenital contamination. |
| **Blood culture** |
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth to Date
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood

| Result | Value |
| | Blood Culture | No Growth to Date |
| **Blood Culture, Peripheral #2** |
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date

No results found.

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. CXR reviewed large right pleural effusion with subjacent atelectasis, found to be empyema. Patient was intubated for acute hypoxic and hypercapnic respiratory failure, admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14. Chest tube removed on 1/21/2024.

 # Cervical discitis/osteomyelitis C4-C6
 # Eval for retropharyngeal and prevertebral abscess vs fluid collection.
No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
- Patient continues to complain of cervical neck pain, no new FND although the pain was not resolving with pain management, patient has been on abx since admission and now on IV abx again.
- ID consulted - Dr. Shao --> recommended CTX and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast if symptomatic since MRI was not completely accurate from motion (plan for 1/29).
- Discussed case w/ IR and they will not be able to drain the area.
- CTX/Flagyl

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
- Wean supplemental oxygen as tolerated
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- s/p Rocephin and Flagyl (1/12-1/21), transition to** **Augmentin then back to CTX/Flagyl when discitis diagnosed.
- PRN Duonebs

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt. Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic.
-Follow up regarding anticoagulation outpatient.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
-Continue Coreg. Will restart lisinopril if tolerated.
-IV Lasix to keep patient euvolemic. Resume oral Lasix tomorrow.
-Strict I's and O's while on Lasix

 # Right ankle/foot pain, likely gout vs disseminated infection.
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Tapped the joint with insufficient aspirate for culture. Recommends continuation of colchicine treatment .
- Continue colchicine.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the

tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray. Lesions have shown much improvement.
- s/p IV acyclovir 400 mg
- s/p oral clotrimazole

 # AKI - resolved
-Creatinine 1.2 OA, from baseline of 0.9.
- Continue to avoid nephrotoxic medications

**# Hyponatremia, likely SIADH d/t lung pathology-worsening **
Fluid restriction to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sod
ium of 17, urine osmolality 152, serum osmolality 265.
- Continue to monitor serum sodium.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#Disposition-
Patient has been evaluated by PT/OT recommended discharge to SNF.

**Resident: **Emuejevuoke Anastasia Umukoro, DO

Attending: Khayyat, Omar Salim Peter, MD

Cosigned by Omar Khayyat, MD at 01/28/2024 11:42 PM PST
Electronically signed by Emuejevuoke Umukoro, DO at 01/28/2024 5:06 PM PST
Electronically signed by Omar Khayyat, MD at 01/28/2024 11:42 PM PST

Associated attestation - Omar Khayyat, MD - 01/28/2024 11:42 PM PST
Formatting of this note might be different from the original.
I have personally examined the patient, reviewed and interpreted the studies and data, discussed the findings and plan of care in detail with the Residents Team, I agree with current plan.

E&M Coding:
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the multidisciplinary team. I have reviewed available medical records, follow up on consult notes, flow sheets, medication record, orders, imaging results, and serial sequential labs.

More than 50 minutes spent in the preparation, coordination, formulation of treatment plan with resident's team, obtaining information personally, reviewing records, consult notes and discuss updates with patient, family and care management team.

MEDICAL DECISION MAKING: This is a high complexity case affecting multiple system. Patient's risk is high given multiple underlying other comorbidities as described above.

I am covering this patient from 8 AM to 10 PM. Everyday between the hours of 10 PM and 8 AM the night hospitalist will be covering this patient and can be reached at 619-600-0746.

Howie Que, DPM - 01/28/2024 1:02 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes

mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/28/24 0902 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Chen, Yu-Chun, DO, 500 mg at 01/22/24 1001
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Khayyat, Omar Salim Peter, MD, 3.125 mg at 01/28/24 0902
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Khayyat, Omar Salim Peter, MD, 2 g at 01/27/24 2057
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throa
t, Before meals & at bedtime, Dammert Coronado, Pedro Manuel, MD, 15 mL at 01/28/24 1140
• enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/28/24 0901
• furosemide (LASIX) injection 40 mg, 40 mg, intraVENOUS, Daily, Khayyat, Omar Salim Peter, MD, 40 mg at 01/28/24 0902
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD, 0.5 mg at 01/28/24 1140
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Chen, Yu-Chun, DO, 25 mg at 01/26/24 1850
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/28/24 0824
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/28/24 0438
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Piatetsky, Yana, MD, 1 patch at 01/27/24 1656
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/24/24 2104
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/28/24 0550 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Shao, Hai, MD, PhD, 500 mg at 01/28/24 0902
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/28/24 0902
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/28/24 0914
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/26/24 1834
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/27/24 2057

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
| **Vitals:** |

| --- |
	01/28/24 1135
BP:	107/54
Pulse:	93
Resp:	30
Temp:	36.9 °C (98.4 °F)
SpO2:	93%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has
underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	01/28/24
0540
SODIUM	mmol/L	**130***
POTASSIUM	mmol/L	4.3
CHLORIDE	mmol/L	**94***
CO2	mmol/L	**36***
BUN	mg/dL	14
CREATININE	mg/dL	0.67
GLUCOSE	mg/dL	97
CALCIUM	mg/dL	8.4

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
01/28/241:03 PM

Electronically signed by Howie Que, DPM at 01/28/2024 1:04 PM PST

Formatting of this note might be different from the original.

PLOF: Independent and homeless. Previously at a homeless shelter in Huntington Beach and then intermittently travels to Mexico. See SW notes for details.

DCP: needs SNF for PT/OT and IV abx (Rocephin for a total of poss 8wks). Will dicuss case further w/ Leadership in am in hopes pt might fit criteria for an Alternative Bed.

Barriers: underfunded and Mcal is for Orange County (however, per HAR notes, pt is in process of switching it to SD).

Electronically signed by Your Scripps Health Team at 01/28/2024 5:34 PM PST

Elaheh S - 01/27/2024 2:31 PM PST

Formatting of this note might be different from the original.

** Occupational Therapy**
[image]

Patient Name: Elizabeth Tina Petruccio
Admission Date: 1/12/2024
Treatment Date: 1/27/2024
Code Status: Full Code
Unit/Room: 0426/01

**Diagnosis: **The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

Social History
Where was pt living / residing: Homeless, unsheltered
(Retired) Type of Residence: Shelter
Assistive Device : (none)
Living Arrangements: Alone
Communication: WNL

Prior Level of Function
Functional Mobility: Independent
ADLs: Independent

[table#1]:
Subjective / Objective / Assessment

[table#2]:
	01/27/24 1431
OT Treatment
OT Treatment Start Time	1416
OT Treatment End Time	1431
OT Treatment Total Minutes	15 minutes
Additional Comments	15 min ADLs, thera act, thera ex
**Subjective **
Subjective	RN cleared pt for OT; pt agreeable.
Interpreter Services
Communication Needs	None
Precautions/Limitations
Precautions	Fall risk
Pain Assessment
Pain Assessment	Verbal Descriptor Scale (VDS)
VDS Pain Score	Moderate Pain (Pain Level 4-6)
Pain Type	Acute pain
Pain Location	Foot
Pain Orientation	Right
Pain Descriptors	Aching
Pain Frequency	Intermittent
Pain Aggravating Factors	Walking
Functional Assessment
Scooting	Stand by Assist
Supine to Sit	Stand by Assist
Sit to Supine	Stand by Assist
Chair to Bed	Stand by Assist

| **Occupational Function** |
ADL/IADL (Y)	ADL
Self Feeding	Independent
Hygiene/Grooming	Set-up
Dressing UE	Set-up
Dressing LE	Minimal Assistance (Subject 75% or more)
(for R foot)
Toileting/Pericare	Contact Guard Assist
Safety Awareness
Safety awareness with ADL/IADL	Good
Safety awareness with functional mobility	Good
Balance
Sitting Static	Good - w/UE
Sitting Dynamic	Fair + w/UE
**Therapeutic Activity **
Therapeutic activities	scooting in supine; transfers; supine<>sit
**Therapeutic Ex
ercise**
Ther Ex	Shoulder
Shoulder	B shoulder AROM thera ex 3 sets/10 rep
**Assessment/ Plan **
Assessment	Pt shows progress w/ being able to complete LB ADLs w/ CGA/Min A. She continues to have pain in R foot and impaired balance that interferes w/ performing ADLs and functional mobility. No s/s distress or SOB noted during OT session.
Plan	Continue w/ POC.
Plan of Care Frequency/Duration	1 time/day;5 times/week;2 weeks
6 Clicks Daily Activity OT
Putting on and taking off regular lower body clothing?	3
Bathing (including washing, rinsing, drying?)	3
Toileting, which includes using toilet, bedpan, or urinal?	3
Putting on and taking off regular upper body clothing?	3
Taking care of personal grooming such as brushing teeth?	3
Eating meals?	4
OT 6 Clicks Score	19
OT Discharge Recommendations
OT Discharge Dispo Recommendations	SNF
OT Safety Concerns	fall risk; homeless

Patient returned/left supine in bed with:
[x] All needs within reach
[x] RN updated on patient's status.
[ ] Other:

Education Provided:
- **Learner: ** [x]Patient** [ ]**Caregiver **[ ]**Family **[ ]** Other:
- **Barriers to Learning:** [x]None [ ]Language [ ]Literacy [ ]Vision [ ]Memory [ ] Hearing [ ]Other:
- **Learning Style:** [x]Verbal [ ]Written [x] Demonstration [ ] Other:

[table#3]:
	***NFT**	**PS**	**VU**	**NVA**	**RD**	**RE**
Role of Occupational Therapy	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Using Nursing Call Light	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Fall Precautions	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Adaptive Equipment / Techniques	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Home Exercise Program	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Positioning/Mobility	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Therapeutic Exercise	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
Other:** **	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
***NFT** = needs further teaching** , PS**= requires practice/supervision, **VU**=verbalized understanding,
NVA=non-verbal acknowledgement; **RD**=return demonstration, **RE**=Refused Education

**Functional Goals: **Short Term Goals
Short Term Goals Due By: 02/03/24
Pt. Will Perform LE Dressing: Yes
Level of Assist: Modified Independent (extra time/device/safety)
Location: EOB
Assistive Device/Equipment: Walker
Pt. Will Perform Toileting: Yes
Level of Assistance: Modified Independent (extra time/device/safety)
Location: Toilet

Assistive Device/Equipment: Walker
Pt. Will Perform Transfers: Yes
Level of Assistance: Modified Independent (extra time/device/safety)
Location: Toilet
Assistive Device/Equipment: Walker

[table#4]:
Plan

Continue with Plan of Care,

[table#5]:
PT/OT/SLP Recommendations

[table#6]:
Flowsheet Row	Most Recent Value
PT Dispo Recommendations	SNF [image] [per PT 1/23, pending progress] filed at 01/27/2024 1023
PT Assistive Device Needs	FWW filed at 01/27/2024 1023
PT Safety Concerns	falls risk, limited social support, limited activity and positional tolerance filed at 01/23/2024 0925
OT Dispo Recommendations	SNF filed at 01/27/2024 1431
OT Safety Concerns	fall risk, homeless filed at 01/27/2024 1431
SLP Safety Concerns	Aspiration Precautions filed at 01/15/2024 1022
SLP Discharge Summary	Dysphagia 3/all liquids filed at 01/15/2024 1022
SLP Discharge Dispo Recommendation	Home filed at 01/15/2024 1022

[table#7]:

Thank you for including Inpatient Rehabilitation in the care of Elizabeth Tina Petruccio.

Elaheh Sobhani, OT
1/27/2024

Electronically signed by Elaheh S at 01/27/2024 3:43 PM PST

Christopher Jones, MD - 01/27/2024 2:17 PM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista
History of Present Illness:
This is hospital day 15 for Elizabeth Tina Petruccio
a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)
** **
** **
Interval Events:
Intubated, chest tube placed, extubated after short time, improvement with chest tube and antibiotics. Chest tube removed 1/20. Thoracentesis 1/25 completed for persistent loculated fluid

1/25 MRI notable for cervical C4-C5 edema with paravertebral collection extending from C2-C7, concern for osteomyelitis discitis without a definitive epidural collection

24 Hour Events:
No acute events overnight.

Patient with extensive notetaking of her incentive spirometer results, neck pain seemed to worsen her efforts as she is s/p dialudid this AM and had improvements in her efforts. Also coached her on breathing timing and inhalation/exhalation, which produced some improvements with IS ~1200 - 1500cc. Denies worsening neck pain, chest discomfort, shortness lightheadedness dizziness or alterations in her mental status

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Intravenous Lines/Tubes/Drains:
All lines and tubes were reviewed on 1/27/2024 and continued to be medically required.
[table#2]:
Patient Devices

[table#3]:
| | [table#3.1]
[table#3.2]
[table#3.3] |
| --- | --- |

[table#3.1]:
Active Devices

[table#3.2]:
	Name	Placement date	Placement time	Site	Days
	Midline Single Lumen 01/14/24 Right Basilic	01/14/24	0215	Basilic	13
	External Urinary Device	—	—	—	14

[table#3.3]:

[table#4]:

Medications:
Scheduled Meds:carvediloL, 3.125 mg, oral, BID
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
[Held by provider] enoxaparin, 40 mg, subcutaneous, Daily
furosemide, 40 mg, intraVENOUS, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, oral, Q12H SCH
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate, oral, TID PRN, Dao, Diep Ngoc, DO
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate, oral, BID PRN, Chen, Yu-Chun, DO
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• hydrOXYzine pamoate, oral, TID PRN, Chen, Yu-Chun, DO
• ipratropium-albuteroL, nebulization, Q4H PRN, Pena, Jose Rafael, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel
, MD

Diet:
[table#5]:

|
|
| --- |
| [table#5.1] |

[table#5.1]:
Dietary Orders
(From admission, onward)

[table#6]:
|
|
| --- |

[table#7]:
| | [table#7.1]
[table#7.2] |
| --- | --- |

[table#7.1]:
	Start			Ordered
	01/22/24 1101		[table#7.1.1]	01/22/24 1100

[table#7.1.1]:
Combination Diet Regular; 1500 mL; Dental Soft Diet effective now
[table#7.1.1.1]

[table#7.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Fluid Restriction - Oral (mL/day):	1500 mL	
Food Consistency:	Dental Soft	

[table#7.2]:

[table#8]:

Allergies on file: Lactose intolerant [lactose]

Review of Systems:
12 organ review of systems was negative other than HPI

Physical Exam:
[table#9]:
Vitals:
	01/27/24 1335
BP:	
Pulse:	
Resp:	
Temp:	
SpO2:	97%

[table#10]:
| Temp: [36.7 °C (98.1 °F)-37.3 °C (99.1 °F)] 37 °C (98.6 °F)
Heart Rate: [94-110] 94
Resp: [18-28] 19
BP: (109-129)/(71-88) 109/71 | SpO2: 97 %
O2 Flow Rate (L/min): 0 L/min | Height: 167.6 cm (5' 6")
Weight: 68 kg (149 lb 14.6 oz)
BMI (Calculated): 24.2 |
| --- | --- | --- |

Cardiac Rhythm: Normal sinus rhythm
In: 240 [P.O.:240]
Out: -

Intake/Output Summary (Last 24 hours) at 1/27/2024 1417
Last data filed at 1/27/2024 0800
[table#11]:
Gross per 24 hour
Intake	245 ml
Output	—
Net	**245 ml**

[table#12]:
Last 5 weights
	01/12/24 1931	01/13/24 0205	01/22/24 0411	01/25/24 1900
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)	65.3 kg (144 lb)	68 kg (149 lb 14.6 oz)

EXAM:
GEN: awake, no apparent distress on room air
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist membranes, clear nares
EYES: PER, anicteric sclera
HEART: RRR, no murmur
LUNGS: clear to auscultation, no wheezing
ABD: soft, NT, ND, + BS
EXT/MSK: no LE edema, 2+ bil LE pulses Swan-neck deformity of multiple fingers and toes bilaterally
NEURO/PSYCH: awake and alert, moving all 4 extremities spontaneously
SKIN: Still with sloughing skin on hands, but looks significantly improved from a few days ago with less redness and cracking

Laboratory Data:

[table#13]:
Results from last 7 days
Lab	Units	01/27/24
0553	01/26/24
0536	01/25/24
0531
WBC	K/mcL	4.8	5.7	4.6
HGB	g/dL	**7.6***	**8.2***	**7.8***
HEMATOCRIT	%	**24.3***	**26.7***	**25.4***
PLT	K/mcL	**452***	**478***	413

[table#14]:
Results from last 7 days
Lab	Units	01/27/24
0553	01/26/24
0536	01/25/24
0531
SODIUM	mmol/L	**129***	**131***	**132***
POTASSIUM	mmol/L	4.4	4.5	4.3
CHLORIDE	mmol/L	**97***	98	**97***
CO2	mmol/L	**32***	**32***	**37***
BUN	mg/dL	14	16	14
CREATININE	mg/dL	0.63	0.67	0.70
CALCIUM	mg/dL	**8.3***	8.4	**8.3***
ALBUMIN	g/dL	**2.7***	**2.8***	**2.6***
BILIRUBIN TOTAL	mg/dL	0.2	0.3	0.2
ALK PHOS	Units/L	71	89	74
ALT	Units/L	16	14	19
AST	Units/L	26	26	27
GLUCOSE	mg/dL	99	100	120

[table#15]:
Results from last 7 days
Lab	Units	01/25/24
0531
LD	Units/L	141

No lab exists for component: "CS30M"

[table#16]:
Lab Results
Component	Value	Date/Time
	BLOODCULT	No Growth to Date	01/26/2024 10:32 AM
	BLOODCULT	No Growth to Date	01/26/2024 10:23 AM

Urinalysis:
[table#17]:
POCT
Componen
t	Value	Date
	PHU	6.0	01/16/2024
	SPECGRAVU	1.025	01/16/2024
	COLORU	Yellow	01/16/2024
	CLARITYU	Clear	01/16/2024
	KETONESU	Negative	01/16/2024
	WBCU	0-2	01/16/2024
	RBCU	0-2	01/16/2024
	PROTEINUR	30 (A)	01/16/2024
	UROBILINOGEN	0.2	01/16/2024
	NITRITEU	Positive (A)	01/16/2024

Cultures:

[table#18]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM

	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth to Date
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Blood Culture, Peripheral #1
	Collection Time: 01/26/24 10:23 AM
	Specimen: Arm, right; Blood
Result	Value
	Blood Culture	No Growth to Date
Blood Culture, Peripheral #2
	Collection Time: 01/26/24 10:32 AM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date

Studies:
No new studies to review

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic re
spiratory failure now resolved , likely secondary to strep pneumo empyema
2. COPD
3. Right sided empyema, status post chest tube and thoracentesis for loculated fluid collection, on antibiotics
4. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes on acyclovir
5. Heart failure, reduced EF
6. Discitis and osteomyelitis of C4-C6, possible high risk for respiratory//
7. Anemia

**Plan: **

1. Agree with ceftriaxone and Flagyl, prolonged course per ID 8 weeks seem reasonable and will also cover her Pulmonary infection.
1. Follow-up blood cultures are no growth to date
2. Continue to monitor incentive spirometry, discussed with patient today continues to chart this
1. Will monitor respiratory muscle strength with incentive spirometer, set goal as 1250 cc which patient was able to achieve with deep breathing. If patient has persistent decline in her respiratory function greater than 500 cc, may need to consider and transfer to ICU for airway watch given cervical spine location of phlegmon
2. - s/p ultrasound guided- thoracentesis of right lung base, 300cc of pleural fluid exudative in nature, pH 7.272 will likely not require repeat chest tube. Will repeat chest x-ray 1/29 and consider repeat ultrasound of chest if right-sided pleural opacity does not improve
3. EF was low before in 2020, 25 to 30%. Likely not worth repeating echo at this time, consider starting on goal-directed medical therapy will defer agents to hospitalist but Coreg is reasonable, consider Entresto as necessary
4. Restart Advair closer to discharge, patient has been requiring routine inhalers . Patient reports having PFTs in the past but I am unable to find any evidence of this in Care Everywhere despite having access to records from other states
5. Continue to monitor nocturnal oxygen saturation has intermittently required low amounts of oxygen overnight may need nocturnal O2 study prior to discharge

#Prophylaxis:
DVT prophylaxis: Resume Lovenox

#CODE STATUS full code

DVT screen: (-) 1/16
Follow-up will plan to see again 1/29 with chest x-ray PA and lateral, please call if needed

Discussed plan of care with primary team.

Disposition:
Per primary team.

Christopher Jones, MD
#802462
Pulmonary & Critical Care Medicine

Total time evaluating patient, speaking with patient/family, reviewing records, laboratory data, creating, coordinating and communicating treatment plan, excluding procedures: 39 minutes

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Christopher Jones, MD at 01/27/2024 2:34 PM PST

Howie Que, DPM - 01/27/2024 1:01 PM PST

Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Pati
ent has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/26/24 1853
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/28/24 0902 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Chen, Yu-Chun, DO, 500 mg at 01/22/24 1001
• carvediloL (COREG) tablet 3.125 mg, 3.125 mg, oral, BID, Khayyat, Omar Salim Peter, MD, 3.125 mg at 01/28/24 0902
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Khayyat, Omar Salim Peter, MD, 2 g at 01/27/24 2057
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Dammert Coronado, Pedro Manuel, MD, 15 mL at 01/28/24 1140
• enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/28/24 0901
• furosemide (LASIX) injection 40 mg, 40 mg, intraVENOUS, Daily, Khayyat, Omar Salim Peter, MD, 40 mg at 01/28/24 0902
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD, 0.5 mg at 01/28/24 1140
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Chen, Yu-Chun, DO, 25 mg at 01/26/24 1850
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/28/24 0824
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/28/24 0438
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Piate
tsky, Yana, MD, 1 patch at 01/27/24 1656
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/24/24 2104
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/28/24 0550 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Shao, Hai, MD, PhD, 500 mg at 01/28/24 0902
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/28/24 0902

• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/28/24 0914
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/26/24 1834
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/27/24 2057

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	01/28/24 1135
BP:	107/54
Pulse:	93
Resp:	30
Temp:	36.9 °C (98.4 °F)
SpO2:	93%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	01/28/24 0540
SODIUM	mmol/L	**130***
POTASSIUM	mmol/L	4.3
CHLORIDE	mmol/L	**94***
CO2	mmol/L	**36***
BUN	mg/dL	14
CREATININE	mg/dL	0.67
GLUCOSE	mg/dL	97
CALCIUM	mg/dL	8.4

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)

Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
01/28/241:01 PM

Electronically signed by Howie Que, DPM at 01/28/2024 1:02 PM PST

Omar Khayyat, MD - 01/27/2024 8:27 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics, she was then started receiving tPA's and dornase through her chest tube for remaining loculations.

Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

1/23/2023
Patient is complaining of neck pain with movement, mostly over the R side but both are painful, this is chronic and has been ongoing for many years, she believes it is headache vs migraine although the patient was able to clearly trace the pain towards the mm of the neck.

1/24
NEON
Patient's foot pain is slightly improving, colchicine dose decreased.
Patient is medically ready to be dced to SNF, unfortunately her insurance is not being accepted by SNFs.

1/25
Patient is having severe headache today, her headache is mostly occipital and seems to be of cervical origin although cannot rule out other causes of headache. Seems to be fluctuating and no clear pattern or triggers for her headache. No fever, no neck rigidity.
Patient's foot pain has been improving, will discontinue colchicine tomorrow if she has no further pain.

1/26
Patient's MRI showing discitis and prevertebral fluid collection, patient hs no headache when awake, she is aware of all the updates, csae discussed with ID - Dr. Shao, ENT - Dr. Hauff and IR.
Will start GDMT as well for HFrEF.

1/27
Patient's Na+ is 129 now, patient with no new headache, she has no dysphagia, sore throat although still has pain when sleeping.
ENT recommended CT w/ contrast if no improvement.
Patient is agreeable to restart CHF meds, she has LE edema and will add IV lasix.
She also wanted to discuss skin peeling/desquamation over the hands only.
Patient reported extensive work up in NJ for autoimmune diseases and will pull her records from her app.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:

Physical Exam
BP 119/80 (BP Location: Left arm, Patient Position: Sitting) | Pulse 99 | Temp 37 °C (98.6 °F) (Oral) | Resp 21 | Ht 1.676 m (5' 6") | Wt 68 kg (149 lb 14.6 oz) | LMP (LMP Unknown) | SpO2 91% | Breastfeeding No | BMI 24.20 kg/m²
I/O: In: 240 [P.O.:240]
Out: -
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. **R chest tube removed. **
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-**Mild swelling and tenderness of the right foot and deformity of the toes.**
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewe
d all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/27/24
0553	01/26/24
0536
WBC	K/mcL	4.8	5.7
HGB	g/dL	**7.6***	**8.2***
MCV	fL	100	100
PLT	K/mcL	**452***	**478***
SODIUM	mmol/L	**129***	**131***
POTASSIUM	mmol/L	4.4	4.5
CHLORIDE	mmol/L	**97***	98
CO2	mmol/L	**32***	**32***
BUN	mg/dL	14	16
CREATININE	mg/dL	0.63	0.67
CALCIUM	mg/dL	**8.3***	8.4
GLUCOSE	mg/dL	99	100
ALBUMIN	g/dL	**2.7***	**2.8***
AST	Units/L	26	26
ALT	Units/L	16	14
BILIRUBIN TOTAL	mg/dL	0.2	0.3
ALK PHOS	Units/L	71	89

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM

	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth to Date
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen

MRI cervical spine with and without contrast

Result Date: 1/26/2024
Impression: Contrast imaging is very limited due to extensive
motion. Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal

fluid, and edema. Findings are concerning for discitis/osteomyelitis at C4-C5. release

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath had been started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. Pleural fluid was found to be empyema and patient was intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. Chest tube removed on 1/21/2024.

 # Acute hypoxic respiratory failure - resolved
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
- Wean supplemental oxygen as tolerated
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- s/p Rocephin and Flagyl (1/12-1/21), transition to** Augmentin then back to CTX/Vanc when discitis diagnosed.**
- PRN Duonebs

 # Cervical discitis/osteomyelitis C4-C6
 # Eval for retropharyngeal and prevertebral abscess vs fluid collection.
**# **No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
- Patient continues to complain of cervical neck pain, no new FND although the pain was not resolving with pain management, patient has been on abx since admission and now on IV abx again.
- ID consulted - Dr. Shao --> recommended CTX and flagyl
- ENT consulted - Dr. Hauff --> repeat CT with contrast if symptomatic since MRI was not completely accurate from motion.
- Discussed case w/ IR and they will not be able to drain the area.
- CTX/Flagyl

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt. Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic.
-Follow up regarding anticoagulation outpatient.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
- Start Coreg and restart lisinopril if tolerated.
- Lasix to keep patient euvolemic.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

 # AKI - resolved
-Creatinine 1.2 OA, from baseline of 0.9.
- Continue to avoid nephrotoxic medications

**# Hyponatremia, likely SIADH d/t lung pathology-worsening **
Fluid restriction to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265.
- Continue to monitor serum sodium.

 # Right ankle/foot pain, likely gout vs disseminated infection.
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule
out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.

- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Tapped the joint with insufficient aspirate for culture. Recommends continuation of colchicine treatment .
- Continue colchicine.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray. Lesions have shown much improvement.
- s/p IV acyclovir 400 mg
- s/p oral clotrimazole

#Disposition-
Patient has been evaluated by PT/OT recommended discharge to SNF.

Khayyat, Omar Salim Peter, MD

Electronically signed by Omar Khayyat, MD at 01/27/2024 12:26 PM PST

Howie Que, DPM - 01/26/2024 4:40 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/22/24 0846
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/26/24 0925 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Chen, Yu-Chun, DO, 500 mg at 01/22/24 1001
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Khayyat, Omar Salim Peter, MD, 2 g at 01/25/24 2147
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W
1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Dammert Coronado, Pedro Manuel, MD, 15 mL at 01/26/24 1323
• [Held by provider] enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at

01/24/24 0855
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD, 0.5 mg at 01/25/24 2059
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Chen, Yu-Chun, DO, 25 mg at 01/23/24 0021
• ipratropium-albuterol (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/26/24 0900
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/26/24 0309
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Piatetsky, Yana, MD, 1 patch at 01/26/24 0310
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/24/24 2104
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/13/24 0030 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• metroNIDAZOLE (FLAGYL) tablet 500 mg, 500 mg, oral, Q12H SCH, Shao, Hai, MD, PhD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/26/24 0925
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/26/24 0052
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/26/24 1323
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/25/24 2144

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	01/26/24 1535
BP:	113/87
Pulse:	103
Resp:	28
Temp:	37 °C (98.6 °F)
SpO2:	94%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
Lower extremity focused physical exam:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservati
ve versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the

first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.

[table#2]:
Results from last 7 days
Lab	Units	01/26/24
0536
SODIUM	mmol/L	**131***
POTASSIUM	mmol/L	4.5
CHLORIDE	mmol/L	98
CO2	mmol/L	**32***
BUN	mg/dL	16
CREATININE	mg/dL	0.67
GLUCOSE	mg/dL	100
CALCIUM	mg/dL	8.4

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)
Empyema of left pleural space (CMS/HCC)
Acute osteomyelitis of cervical spine (CMS/HCC)

Howie Que, DPM
01/26/244:40 PM

Electronically signed by Howie Que, DPM at 01/26/2024 4:40 PM PST

Christopher Jones, MD - 01/26/2024 12:27 PM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista
History of Present Illness:
This is hospital day 14 for Elizabeth Tina Petruccio a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)
** **

Interval Events:
Intubated, chest tube placed, extubated after short time, improvement with chest tube and antibiotics. Chest tube removed 1/20. Thoracentesis 1/25 completed for persistent loculated fluid

24 Hour Events:
Patient obtained MRI for ongoing headaches yesterday, notable for cervical C4-C5 edema with paravertebral collection extending from C2-C7, concern for osteomyelitis discitis without a definitive epidural collection.

On my exam today patient is in good spirits, actually laughs and says "I am falling apart" when we talk with her regarding her possible spine infection. Otherwise she denies any changes in her breathing, dyspnea, chest pain or chest tightness, changes in her voice, double vision

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Intravenous Lines/Tubes/Drains:
All lines and tubes were reviewed on 1/26/2024 and continued to be medically required.
[table#2]:
Patient Devices

[table#3]:
| | [table#3.1]

[table#3.2]
[table#3.3] |
| --- | --- |

[table#3.1]:
Active Devices

[table#3.2]:
	Name	Placement date	Placement time	Site	Days
	Midline Single Lumen 01/14/24 Right Basilic	01/14/24	0215	Basilic	12
	External Urinary Device	—	—	—	13

[table#3.3]:

[table#4]:

Medications:
Scheduled Meds:cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
[Held by provider] enoxaparin, 40 mg, subcutaneous, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID
vancomycin, 750 mg, intraVENOUS, Q8H

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate, oral, TID PRN, Dao, Diep Ngoc, DO
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily
PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate, oral, BID PRN, Chen, Yu-Chun, DO
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• hydrOXYzine pamoate, oral, TID PRN, Chen, Yu-Chun, DO
• ipratropium-albuteroL, nebulization, Q4H PRN, Pena, Jose Rafael, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Diet:
[table#5]:
|
|
| --- |
| [table#5.1] |

[table#5.1]:
Dietary Orders
(From admission, onward)

[table#6]:
|

|
| --- |

[table#7]:
| | [table#7.1]
[table#7.2] |
| --- | --- |

[table#7.1]:
	Start			Ordered
	01/22/24 1101		[table#7.1.1]	01/22/24 1100

[table#7.1.1]:
Combination Diet Regular; 1500 mL; Dental Soft Diet effective now
[table#7.1.1.1]

[table#7.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Fluid Restriction - Oral (mL/day):	1500 mL	
Food Consistency:	Dental Soft	

[table#7.2]:

[table#8]:

Allergies on file: Lactose intolerant [lactose]

Review of Systems:
12 organ review of systems was negative other than HPI

Physical Exam:
[table#9]:
Vitals:
	01/26/24 1200
BP:	123/85
Pulse:	99
Resp:	16
Temp:	
SpO2:	92%

[table#10]:
| Temp: [36.4 °C (97.5 °F)-37.1 °C (98.8 °F)] 36.9 °C (98.4 °F)
Heart Rate: [90-117] 99
Resp: [16-23] 16
BP: (115-136)/(74-91) 123/85
FiO2 (%): [24 %] 24 % | SpO2: 92 %
FiO2 (%): 24 %
O2 Flow Rate (L/min): 1 L/min | Height: 167.6 cm (5' 6")
Weight: 68 kg (149 lb 14.6 oz)
BMI (Calculated): 24.2 |
| --- | --- | --- |
Cardiac Rhythm: Normal sinus rhythm
In: 990.2 [P.O.:720; I.V.:20; IV Piggyback:250.2]
Out: 300 [Other:300]

Intake/Output Summary (Last 24 hours) at 1/26/2024 1227
Last data filed at 1/26/2024 0725
[table#11]:
Gross per 24 hour
Intake	990.17 ml

| Output | — |
| **Net** | **990.17 ml** |

[table#12]:
Last 5 weights
	01/12/24 1931	01/13/24 0205	01/22/24 0411	01/25/24 1900
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)	65.3 kg (144 lb)	68 kg (149 lb 14.6 oz)

FiO2 (%): [24 %] 24 %

EXAM:
GEN: awake, no apparent distress on room air
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist membranes, clear nares
EYES: PER, anicteric sclera
HEART: RRR, no murmur
LUNGS: clear to auscultation, no wheezing
ABD: soft, NT, ND, + BS
EXT/MSK: no LE edema, 2+ bil LE pulses
NEURO/PSYCH: awake and alert, moving all 4 extremities spontaneously
SKIN: no rashes, no skin lesions

Laboratory Data:

[table#13]:
Results from last 7 days
Lab	Units	01/26/24
0536	01/25/24
0531	01/24/24
0548
WBC	K/mcL	5.7	4.6	4.8
HGB	g/dL	**8.2***	**7.8***	**8.0***
HEMATOCRIT	%	**26.7***	**25.4***	**26.3***
PLT	K/mcL	**478***	413	364

[table#14]:
Results from last 7 days

| Lab | Units | 01/26/24
0536 | 01/25/24
0531 | 01/24/24
0548 |
SODIUM	mmol/L	**131***	**132***	**132***
POTASSIUM	mmol/L	4.5	4.3	4.9
CHLORIDE	mmol/L	98	**97***	98
CO2	mmol/L	**32***	**37***	**37***
BUN	mg/dL	16	14	16
CREATININE	mg/dL	0.67	0.70	0.70
CALCIUM	mg/dL	8.4	**8.3***	**8.0***
ALBUMIN	g/dL	**2.8***	**2.6***	**2.6***
BILIRUBIN TOTAL	mg/dL	0.3	0.2	0.2
ALK PHOS	Units/L	89	74	74
ALT	Units/L	14	19	17
AST	Units/L	26	27	25
GLUCOSE	mg/dL	100	120	94

[table#15]:
Results from last 7 days
Lab	Units	01/25/24
0531
LD	Units/L	141

No lab exists for component: "CS30M"

[table#16]:
Lab Results
Component	Value	Date/Time
	BLOODCULT	No Growth Final	01/12/2024 09:48 PM
	BLOODCULT	No Growth Final	01/12/2024 09:30 PM

Urinalysis:
[table#17]:
POCT
Component	Value	Date
	PHU	6.0	01/16/2024
	SPECGRAVU	1.025	01/16/2024
	COLORU	Yellow	01/16/2024
	CLARITYU	Clear	01/16/2024
	KETONESU	Negative	01/16/2024
	WBCU	0-2	01/16/2024
	RBCU	0-2	01/16/2024
	PROTEINUR	30 (A)	01/16/2024
	UROBILINOGEN	0.2	01/16/2024
	NITRITEU	Positive (A)	01/16/2024

Cultures:

[table#18]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain

	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value

| | Urine Culture | |
| | | Multiple organisms present, probable skin/urogenital contamination. |
| **MRSA Surveillance** |
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth to Date
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen

Studies:
Reviewed MRI cervical spine without contrast report noting abnormal edema at C4-C6 with paravertebral collection extending from C2-C7 concerning for osteomyelitis with discitis Splane to retropharyngeal paravertebral soft tissues without a definitive epidural collection.

Reviewed chest x-ray after thoracentesis showing right effusion with compressive change and volume loss

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure now resolved , likely secondary to strep pneumo empyema
2. COPD
3. Right sided empyema, status post chest tube and thoracentesis for loculated fluid collection, on antibiotics
4. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes on acyclovir
5. Heart failure, reduced EF
6.

**Plan: **

1. Agree with broadening IV antibiotics given concern for discitis/osteomyelitis with possible paravertebral phlegmon, recommend

IR versus neurosurgery consultation, ID consultation, MRI spine with and without contrast
1. Repeat blood cultures, have held Lovenox in case intervention planned
2. Communicate with primary team, nursing staff, and patient. Will monitor respiratory muscle strength with incentive spirometer, set goal as 1250 cc this morning which patient was able to achieve with deep breathing. If patient has persistent decline in her respiratory function greater than 300 cc, may need to consider SNF and transfer to ICU for airway watch given cervical spine location of phlegmon
2. - s/p ultrasound guided- thoracentesis of right lung base, 300cc of pleural fluid exudative in nature, pH 7.272 will likely not require repeat chest tube. Will repeat chest x-ray in a few days and consider repeat ultrasound of chest if right-sided pleural opacity does not improve
3. EF was low before in 2020, 25 to 30%. Likely not worth repeating echo at this time, consider starting on goal-directed medical therapy will defer agents to hospitalist
4. Duration of antibiotics to be determined pending evaluation of cervical spine.
5. Restart Advair closer to discharge, patient has been requiring routine inhalers

#Prophylaxis:
DVT prophylaxis: On hold for possible procedure

#CODE STATUS full code

DVT screen: Negative on 1/16
Follow-up will follow

Discussed plan of care with primary team.

Disposition:
Per primary team.

Christopher Jones, MD
#802462
Pulmonary & Critical Care Medicine

Total time evaluating patient, speaking with patient/family, reviewing records, laboratory data, creating, coordinating and communicating treatment plan, excluding procedures: 37 minutes

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a moderate complexity case affecting the respiratory system.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of th
e dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Christopher Jones, MD at 01/26/2024 1:41 PM PST

Omar Khayyat, MD - 01/26/2024 7:42 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics, she was then started receiving tPA's and dornase through her chest tube for remaining loculations.

Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

1/23/2023
Patient is complaining of neck pain with movement, mostly over the R side but both are painful, this is chronic and has been ongoing for many years, she believes it is headache vs migraine although the patient was able to clearly trace the pain towards the mm of the neck.

1/24
NEON
Patient's foot pain is slightly improving, colchicine dose decreased.
Patient is medically ready to be dced to SNF, unfortunately her insurance is not being accepted by SNFs.

1/25
Patient is having severe headache today, her headache is mostly occipital and seems to be of cervical origin although cannot rule out other causes of headache. Seems to be fluctuating and no clear pattern or triggers for her headache. No fever, no neck rigidity.
Patient's foot pain has been improving, will discontinue colchicine tomorrow if she has no further pain.

1/26
Patient's MRI showing discitis and prevertebral fluid collection, patient hs no headache when awake, she is aware of all the updates, csae discussed with ID - Dr. Shao, ENT - Dr. Hauff and IR.
Will start GDMT as well for HFrEF.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 120/89 (BP Location: Left arm, Patient Position: Lying) | Pulse 101 | Temp 36.7 °C (98.1 °F) (Oral) | Resp 17 | Ht 1.676 m (5' 6") | Wt 68 kg (149 lb 14.6 oz) | LMP (LMP Unknown) | SpO2 96% | Breastfeeding No | BMI 24.20 kg/m²
I/O: In: 990.2 [P.O.:720; I.V.:20; IV Piggyback:250.2]
Out: 300 [Other:300]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. **R chest tube removed. **
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-**Mild swelling and tenderness of the right foot and deformity of the toes**
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs in
clude:

[table#5]:
Results from last 7 days
Lab	Units	01/26/24
0536	01/25/24
0531	01/24/24
0548
WBC	K/mcL	--	4.6	4.8
HGB	g/dL	--	**7.8***	**8.0***
MCV	fL	--	100	**102***
PLT	K/mcL	--	413	364
SODIUM	mmol/L	**131***	**132***	**132***
POTASSIUM	mmol/L	4.5	4.3	4.9
CHLORIDE	mmol/L	98	**97***	98

CO2	mmol/L	**32***	**37***	**37***
BUN	mg/dL	16	14	16
CREATININE	mg/dL	0.67	0.70	0.70
CALCIUM	mg/dL	8.4	**8.3***	**8.0***
GLUCOSE	mg/dL	100	120	94
ALBUMIN	g/dL	**2.8***	**2.6***	**2.6***
AST	Units/L	26	27	25
ALT	Units/L	14	19	17
BILIRUBIN TOTAL	mg/dL	0.3	0.2	0.2
ALK PHOS	Units/L	89	74	74

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final

| | Gram Stain | Moderate WBCs |
| | Gram Stain | No organisms seen |
| **Strep pneumo antigen, urine** |
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen

MRI cervical spine without contrast

Result Date: 1/25/2024
IMPRESSION: 1. A
bnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation. 2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels. Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024. release 1419 A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

MRI brain without contrast

Result Date: 1/25/2024
IMPRESSION: 1. No acute intracranial findings. 2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change. 3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process. release

X-ray chest single frontal view

Result Date: 1/25/2024
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated release

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath had been started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. Pleural fluid was found to be empyema and patient was intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. Chest tube removed on 1/21/2024.

 # Acute hypoxic respiratory failure
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest

CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
- Wean supplemental oxygen as tolerated
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- s/p Rocephin and Flagyl (1/12-1/21), transition to** Augmentin then back to CTX/Vanc when discitis diagnosed.**
- PRN Duonebs

 # Cervical discitis/osteomyelitis C4-C6
 # Eval for retropharyngeal and prevertebral abscess vs fluid collection.
**# **No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.
- Patient continues to complain of cervical neck pain, no new FND although the pain was not resolving with pain management, patient has been on abx since admission and now on IV abx again.
- ID consulted - Dr. Koura
- CTX/Vanc
- MRI 1/25 with the findings above and radiology recommended repeat with contrast --> Pending.

** 6194009779**

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt. Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic.
-Follow up regarding anticoagulation outpatient.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
\
- Holding Coreg and lisinopril given soft BP
- When appropriate initiate low-dose carvedilol.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

 # AKI - resolved
-Creatinine 1.2 OA, from baseline of 0.9.
- Continue to avoid nephrotoxic medications

**# Hyponatremia, likely SIADH d/t lung pathology-worsening **
Fluid restriction to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265.
- Continue to monitor serum sodium.

 # Right ankle/foot pain, likely gout
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Tapped the joint with insufficient aspirate for culture. Recommends continuation of colchicine treatment .
- Continue colchicine.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray. Lesions have shown much improvement.
- Continue IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.
- Continue oral clotrimazole

#Disposition-
Patient has been evaluated by PT/OT recommended discharge to SNF.

Khayyat, Omar Salim Peter, MD

Electronically signed by Omar Khayyat, MD at 01/27/2024 12:06 AM PST

Irene G - 01/25/2024 10:42 PM PST

Formatting of this note might be different from the original.

Pharmacokinetic Consult – Vancomycin Dosing Service
Initiated on: 1/25/24
Clinical Pharmacist Note

Elizabeth Tina Petruccio is a 59 y.o. female has been started on vancomycin for Discitis/Osteomylitis. AUC/MIC goal is 400-600 mg·hr/L.

Relevant clinical data:
[table#1]:
Anti-infectives (From admission, onward)

[table#2]:
	Start			Dose/Rate	Route	Frequency	Ordered	Stop
	01/26/24 0800		[table#2.1]	750 mg
180 mL/hr over 90 Minutes	intraVENOUS	Every 8 hours	01/25/24 2248	
	01/25/24 2300		[table#2.2]	20 mg/kg × 68 kg
125 mL/hr over 120 Minutes	intraVENOUS	Once	01/25/24 2242	
	01/25/24 2145		[table#2.3]	2 g
120 mL/hr	intraVENOUS	Every 24 hours	01/25/24 2118	02/01/24 2144
	01/16/24 0900		[table#2.4]	10 mg	mucous membrane	3 times daily	01/22/24 2156	01/26/24 0859

[table#2.1]:
vancomycin (VANCOCIN) 750 mg in sodium chloride 0.9 % (NS) 250 mL Vial2Bag

[table#2.2]:
vancomycin (VANCOCIN) IVPB 1,250 mg in sodium chloride 0.9 % (NS) 250 mL (Premix)

[table#2.3]:
cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g

[table#2.4]:
clotrimazole (MYCELEX) troche 10 mg
[table#3]:

[table#4]:
Ht Readings from Last 1 Encounters:
01/25/24	1.676 m (5' 6")

68 kg (149 lb 14.6 oz)

[table#5]:
Results from last 7 days
Lab	Units	01/25/24
0531	01/24/24
0548	01/23/24
0544
CREATININE	mg/dL	0.70	0.70	0.80
BUN	mg/dL	14	16	16
WBC	K/mcL	4.6	4.8	4.2

Estimated Creatinine Clearance: 81 mL/min (by C-G formula based on SCr of 0.7 mg/dL).

Temp (24hrs), Avg:37.1 °C (98.7 °F
), Min:36.7 °C (98.1 °F), Max:37.4 °C (99.3 °F)

I/O last 3 completed shifts:

In: 1690 [P.O.:1680; I.V.:10]
Out: 300 [Other:300]

Microbiology culture results:
[table#6]:
Recent Results (from the past 168 hour(s))
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value	Ref Range
	Staph Screen culture		
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value	Ref Range
	Gram Stain	Many WBCs	
	Gram Stain	No organisms seen	

MRSA Nares Screen: Not Indicated - Patient is not receiving treatment for pneumonia

Assessment
Renal function:
Serum creatinine stable

Pharmacokinetics:
Levels not yet obtained

Plan
After receiving a loading dose of 1250 mg IV x1, patient will be initiated on a regimen of 750 mg IV q8h, to which pharmacokinetic modeling predicts a vancomycin half life of 7.5 hours and an AUC/MIC of 558 mg·hr/L

Will defer serum sampling at this time pending clinical course.
Pharmacist will continue to follow the patient's culture results and clinical progress daily and adjust the dose as needed.

1/25/2024

Electronically signed by Irene G at 01/25/2024 10:52 PM PST

Howie Que, DPM - 01/25/2024 12:54 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• acyclovir (ZOVIRAX) suspension 400 mg, 400 mg, oral, Q8H SCH, Khoo, Jennifer, PharmD, 400 mg at 01/25/24 0634
• amoxicillin-clavulanate (AUGMENTIN) 875-125 mg per tablet 1 tablet, 1 tablet, oral, BID, Dao, Diep Ngoc, DO, 1 tablet at 01/25/24 0833
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/22/24 0846
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/25/24 0833 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lact
ulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Chen, Yu-Chun, DO, 500 mg at 01/22/24 1001
• clotrimazole (MYCELEX) troche 10 mg, 10 mg, mucous membrane, TID, Dao, Diep Ngoc, DO, 10 mg at 01/25/24 0833
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Khayyat, Omar Salim Peter, MD, 0.6 mg at 01/25/24 0833
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Dammert Coronado, Pedro Manuel, MD, 15 mL at 01/25/24 1205
• [Held by provider] enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/24/24 0855
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD, 0.5 mg at 01/24/24 0522
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Chen, Yu-Chun, DO, 25 mg at 01/23/24 0021
• ipratropium-albuterol (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/25/24 0650
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/25/24 0445
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Piatetsky, Yana, MD, 1 patch at 01/25/24 0445
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/24/24 2104
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/13/24 0030 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/25/24 0833
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/25/24 0839
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/25/24 0023
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/25/24 0022

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	01/25/24 1109
BP:	119/89
Pulse:	95
Resp:	18
Temp:	37.4 °C (99.3 °F)
SpO2:	94%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
Lower extremity focused physical exam:

MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joi
nt. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	01/25/24
0531
SODIUM	mmol/L	**132***
POTASSIUM	mmol/L	4.3
CHLORIDE	mmol/L	**97***
CO2	mmol/L	**37***
BUN	mg/dL	14
CREATININE	mg/dL	0.70
GLUCOSE	mg/dL	120
CALCIUM	mg/dL	**8.3***

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)

Howie Que, DPM
01/25/2412:54 PM

Electronically signed by Howie Que, DPM at 01/25/2024 12:54 PM PST

Ikramah Latif, DO - 01/25/2024 12:43 PM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista

Chief complain:
Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

History of Present Illness:
This is hospital day 13 for Elizabeth Tina Petruccio a 59 y/o woman admitted with an empyema . Intubated, chest tube placed, improvement with chest tube and antibiotics. Chest tube removed 1/20.

Subjective:
Evaluated at bedside
No overnight events
Thoracentesis procedure today, consent signed after discussing risks and benefits. Patient agrees.

Past Medical History:
[table#1]:

Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
Scheduled Meds:acyclovir, 400 mg, oral, Q8H SCH
amoxicillin-clavulanate, 1 tablet, oral, BID
clotrimazole, 10 mg, mucous membrane, TID
colchicine, 0.6 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
[Held by provider] enoxaparin, 40 mg, subcutaneous, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate, oral, TID PRN, Dao, Diep Ngoc, DO
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate, oral, BID PRN, Chen, Yu-Chun, DO
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D
5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• hydrOXYzine pamoate, oral, TID PRN, Chen, Yu-Chun, DO
• ipratropium-albuteroL, nebulization, Q4H PRN, Pena, Jose Rafael, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Allergies on file: Lactose intolerant [lactose]

Review of Systems:
10 organ review of systems was negative other than what mentioned in HPI

**Family History: **
[table#2]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Social History:
[table#3]:
Social History

[table#4]:
Socioeconomic History
•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	Not on file
•	Years of education:	Not on file

| • | Highest education level: | Not on file |
| Occupational History |
| • | Not on file |
| Tobacco Use |
| • | Smoking status: | Former |
| • | Smokeless tobacco: | Never |
| Substance and Sexual Activity |
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#5]:
Social Determinants of Health

[table#6]:
Financial Resource Strain: Not on file
Food Insecurity: At Risk (1/19/2024)
	Food Insecurity
	•	In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Yes
Transportation Needs: At Risk (1/19/2024)
	Transportation Needs
	•	In the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Yes
Physical Activity: Not on file
Stress: Not on file
Social Connections: Not on file
Intimate Partner Violence: Not on file
Housing Stability: High Risk (1/19/2024)
	Housing Stability
	•	What is your living situation today?: Do not have a stable place to live

Physical Exam:
[table#7]:
Vitals:
	01/25/24 1109
BP:	119/89
Pulse:	95
Resp:	18
Temp:	37.4 °C (99.3 °F)
SpO2:	94%

[table#8]:
| Temp: [36.7 °C (98.1 °F)-37.4 °C (99.3 °F)] 37.4 °C (99.3 °F)
Heart Rate: [74-113] 95
Resp: [14-23] 18
BP: (119-127)/(76-89) 119/89
FiO2 (%): [21 %] 21 % | SpO2: 94 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 0 L/min | Height: 170.2 cm (5' 7")
Weight: 65.3 kg (144 lb)
BMI (Calculated): 22.5 |
| --- | --- | --- |
Cardiac Rhythm: Normal sinus rhythm
In: 1210 [P.O.:1200; I.V.:10]
Out: -

Intake/Output Summary (Last 24 hours) at 1/25/2024 1243
Last data filed at 1/25/2024 0716
[table#9]:
Gross per 24 hour
Intake	1210 ml

| Output | — |
| **Net** | **1210 ml** |

[table#10]:
Last 5 weights
	01/12/24 1931	01/13/24 0205	01/22/24 0411
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)	65.3 kg (144 lb)

FiO2 (%): [21 %] 21 %

EXAM:
GEN: awake, in mld distress
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist mucous membranes, oral ulcers
HEART: RRR, no murmur
LUNGS: lungs clear to auscultation but diminished posterior right lower lobe
ABD: soft, NT, ND,
EXT/MSK:
no LE edema, ulnar deviation of bilateral hands and feet,
NEURO: awake and alert, moving all 4 ext to command
Skin/joints: Swan-neck deformity of multiple fingers and toes bilaterally

**Laboratory Data: ** Personally Reviewed

**Imaging: ** Personally Reviewed

Chest x-rays reviewed over several days, notable increasing loculated pleural effusion on right side that has progressed since interval removal of chest tube

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure
2. COPD
3. Right sided empyema, improving
4. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes
5. Heart failure, reduced EF

Recommendations:
- s/p ultrasound guided- thoracentesis of right lung base, 300cc of pleural fluid
- repeat chest x-ray, follow up, may resume lovenox if no pneumothorax
- Continue Augmentin to complete a total of 4 weeks of antibiotic therapy
- Restart Advair closer to discharge, patient has been requiring routine inhalers

Ikramah Latif, DO, 1/25/2024, 12:43 PM
Pulmonary and Critical Care Medicine
#8086509

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system. Patient's risk is high.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Cosigned by Christopher Jones, MD at 01/25/2024 1:26 PM PST
Electronically signed by Ikramah Latif, DO at 01/25/2024 12:50 PM PST
Electronically signed by Christopher Jones, MD at 01/25/2024 1:26 PM PST

Associated attestation - Christopher Jones, MD - 01/25/2024 1:26 PM PST
Formatting of this note might be different from the original.
Patient was seen, evaluated, and examined independently by the Resident.

I have also personally performed a face-to-face diagnostic evaluation on this patient. I have reviewed and agree with

the care plan documented by the Resident except as noted below.

My plan is as follows: Patient doing well, surprised by amount of fluid we were able to remove with procedure. Will follow-up chest x-ray today, lung ultrasound after procedure did not show evidence of pneumothorax. Will plan on repeat chest x-ray at a later date to assess for repeat accumulation of pleural fluid. Continue antibiotics

Christopher W Jones, MD
Pulmonary and Critical Care Attending

Total time evaluating patient, speaking with patient/family, reviewing records, laboratory data, creating and communicating treatment plan, excluding procedures: 26 minutes

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a moderate complexity case affecting the respiratory system.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Sandra S - 01/25/2024 9:30 AM PST
Formatting of this note might be different from the original.

[table#1]:
	01/25/24 0909
Oxygen Therapy/Pulse Ox
Oxygen Therapy	None (Room air)
O2 Delivery Method	Other (Comment)
O2 Flow Rate (L/min)	0 L/min
FiO2 (%)	21 %
SpO2	96 %
Patient Activity	At rest
$Oxygen Initiated/Discontinued	Discontinued
$ Pulse Oximetry Spot Check	Yes
Vital Signs
Heart Rate	113
Resp	18
P/F Calculation
P/F Ratio	0

Correction to be noted. Pt is **waring** her Nasal cannula. And Flow meter was set at 0.5 lpm

When I followed the tubing. Nasal cannula was not connected.

AS stand. Pt is on Room air. Spo2 of 92% RR 16 at rest.
Sandra M. Sandoval-Medina, RT, 1/25/2024, 8:57 AM

Electronically signed by Sandra S at 01/25/2024 9:31 AM PST

Omar Khayyat, MD - 01/25/2024 8:19 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and

was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics, she was then started receiving tPA's and dornase through her chest tube for remaining loculations.

Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

1/23/2023
Patient is complaining of neck pain with movement, mostly over the R side but both are painful, this is chronic and has been ongoing for many years, she believes it is headache vs migraine although the patient was able to clearly trace the pain towards the mm of the neck.

1/24
NEON
Patient's foot pain is slightly improving, colchicine dose decreased.
Patient is medically ready to be dced to SNF, unfortunately her insurance is not being accepted by SNFs.

1/25
Patient is having severe headache today, her headache is mostly occipital and seems to be of cervical origin although cannot rule out other causes of headache. Seems to be fluctuating and no clear pattern or triggers for her headache. No fever, no neck rigidity. Patient's foot pain has been improving, will discontinue colchicine tomorrow if she has no further pain.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 122/81 (BP Location: Left arm, Patient Position: Sitting) | Pulse 102 | Temp 37.3 °C (99.1 °F) (Oral) | Resp 20 | Ht 1.702 m (5' 7") | Wt 65.3 kg (144 lb) | LMP (LMP Unknown) | SpO2 91% | Breastfeeding No | BMI 22.55 kg/m²
I/O: In: 1210 [P.O.:1200; I.V.:10]
Out: -
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. **R chest tube removed. **
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-**Mild swelling and tenderness of the right foot and deformity of the toes**
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/25/24
0531	01/24/24
0548
WBC	K/mcL	4.6	4.8
HGB	g/dL	**7.8***	**8.0***
MCV	fL	100	**102***
PLT	K/mcL	413	364
SODIUM	mmol/L	**132***	**132***
POTASSIUM	mmol/L	4.3	4.9

| CHLORIDE | mmol/L | **97*** | 98 |
| CO2 | mmol/L | **37**
* | **37*** |
BUN	mg/dL	14	16
CREATININE	mg/dL	0.70	0.70
CALCIUM	mg/dL	**8.3***	**8.0***
GLUCOSE	mg/dL	120	94
ALBUMIN	g/dL	**2.6***	**2.6***
AST	Units/L	27	25
ALT	Units/L	19	17
BILIRUBIN TOTAL	mg/dL	0.2	0.2
ALK PHOS	Units/L	74	74

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum

Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus

No results found.

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath had been started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. Pleural fluid was found to be empyema and patient was intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. Chest tube removed on 1/21/2024.

**# Acute hypoxic respiratory
failure**
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
- Wean supplemental oxygen as tolerated
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- s/p Rocephin and Flagyl (1/12-1/21), transition to** Augmentin x 4 weeks.**
- PRN Duonebs

**# Severer occiptal and cervical headaches, **patient belies that this is migraine although her headache description is not completely consistent, no clear spinal tenderness, no FND and no other red flags.
- Check MRI brain and cervical spine to eval since patient seems to be debilitated by her headache and unable to function.

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt. Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic.
-Follow up regarding anticoagulation outpatient.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
- Holding Coreg and lisinopril given soft BP
- When appropriate initiate low-dose carvedilol.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up

with the homelessness response center

 # AKI - resolved
-Creatinine 1.2 OA, from baseline of 0.9.
- Continue to avoid nephrotoxic medications

**# Hyponatremia, likely SIADH d/t lung pathology-worsening **
Fluid restriction to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265.
- Continue to monitor serum sodium.

 # Right ankle/foot pain, likely gout
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Tapped the joint with insufficient aspirate for culture. Recommends continuation of colchicine treatment .
- Continue colchicine.

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray. Lesions have shown much improvement.
- Continue IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.
- Continue oral clotrimazole

#Disposition-
Patient has been evaluated by PT/OT recommended discharge to SNF.

Khayyat, Omar Salim Peter, MD

Electronically signed by Omar Khayyat, MD at 01/25/2024 2:51 PM PST

Howie Que, DPM - 01/24/2024 5:52 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightb
earing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes
mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications

• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• acyclovir (ZOVIRAX) suspension 400 mg, 400 mg, oral, Q8H SCH, Khoo, Jennifer, PharmD, 400 mg at 01/24/24 1359
• amoxicillin-clavulanate (AUGMENTIN) 875-125 mg per tablet 1 tablet, 1 tablet, oral, BID, Dao, Diep Ngoc, DO, 1 tablet at 01/24/24 0855
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/22/24 0846
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/24/24 0855 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Chen, Yu-Chun, DO, 500 mg at 01/22/24 1001
• clotrimazole (MYCELEX) troche 10 mg, 10 mg, mucous membrane, TID, Dao, Diep Ngoc, DO, 10 mg at 01/24/24 1400
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Khayyat, Omar Salim Peter, MD, 0.6 mg at 01/24/24 0857
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Dammert Coronado, Pedro Manuel, MD, 15 mL at 01/24/24 1739
• [Held by provider] enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/24/24 0855
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD, 0.5 mg at 01/24/24 0522
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Chen, Yu-Chun, DO, 25 mg at 01/23/24 0021
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/24/24 1705
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/23/24 2024
• lidoca
ine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/24/24 0522
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Piatetsky, Yana, MD, 1 patch at 01/23/24 2250
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/23/24 2250
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/13/24 0030 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/24/24 0855
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/24/24 1359
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/23/24 0154
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/22/24 2110

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	01/24/24 1712
BP:	
Pulse:	
Resp:	22
Temp:	
SpO2:	97%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
Lower extremity focused physical exam:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot

severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	01/24/24
0548
SODIUM	mmol/L	**132***
POTASSIUM	mmol/L	4.9
CHLORIDE	mmol/L	98
CO2	mmol/L	**37***
BUN	mg/dL	16
CREATININE	mg/dL	0.70
GLUCOSE	mg/dL	94
CALCIUM	mg/dL	**8.0***

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)

Howie Que, DPM
01/24/245:52 PM

Electronically signed by Howie Que, DPM at 01/24/2024 5:52 PM PST

Ikramah Latif, DO - 01/24/2024 1:03 PM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista

Chief complain:
Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt.
Has history of COPD, CHF, AFIB)

History of Present Illness:
This is hospital day 12 for Elizabeth Tina Petruccio a 59 y/o woman admitted with an empyema . Intubated, chest tube placed, improvement with chest tube and antibiotics. Chest tube removed 1/20.

Subjective:
No acute overnight events, denies SOB/dyspnea.
Evaluated at bedside satting 98% on room air, not on oxygen
Complaining of right foot pain 2/2 to gout, hand pain, and headache.

Past Medical History:
[table#1]:
Past Medical History:

Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
Scheduled Meds:acyclovir, 400 mg, oral, Q8H SCH
amoxicillin-clavulanate, 1 tablet, oral, BID
clotrimazole, 10 mg, mucous membrane, TID
colchicine, 0.6 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
[Held by provider] enoxaparin, 40 mg, subcutaneous, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
lidocaine-menthol, 1 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate, oral, TID PRN, Dao, Diep Ngoc, DO
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate, oral, BID PRN, Chen, Yu-Chun, DO
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• hydrOXYzine pamoate, oral, TID PRN, Chen, Yu-Chun, DO
• ipratropium-albuteroL, nebulization, Q4H PRN, Pena, Jose Rafael, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Allergies on file: Lactose intolerant [lactose]

Review of Systems:
10 organ review of systems was negative other than what mentioned in HPI

**Family History: **
[table#2]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Social History:
[table#3]:
Social History

[table#4]:
Socioeconomic History
•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	Not on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file

| Tobacco Use |
| • | Smoking status: | Former |
| • | Smokeless tobacco: | Never |
| Substance and Sexual Activity |
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#5]:
Social Determinants of Health

[table#6]:
| Financial Resource Strain: Not on f
ile |
| --- |
| Food Insecurity: At Risk (1/19/2024) |
| | Food Insecurity |
| | • | In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Yes |
| Transportation Needs: At Risk (1/19/2024) |
| | Transportation Needs |
| | • | In the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Yes |
| Physical Activity: Not on file |
| Stress: Not on file |
| Social Connections: Not on file |
| Intimate Partner Violence: Not on file |
| Housing Stability: High Risk (1/19/2024) |
| | Housing Stability |
| | • | What is your living situation today?: Do not have a stable place to live |

Physical Exam:
[table#7]:
Vitals:
	01/24/24 1100
BP:	
Pulse:	
Resp:	
Temp:	36.8 °C (98.2 °F)
SpO2:	

[table#8]:
| Temp: [36.6 °C (97.9 °F)-37.1 °C (98.8 °F)] 36.8 °C (98.2 °F)
Heart Rate: [82-106] 95
Resp: [12-28] 18
BP: (102-124)/(58-89) 109/84 | SpO2: 98 %
O2 Flow Rate (L/min): 0.5 L/min | Height: 170.2 cm (5' 7")
Weight: 65.3 kg (144 lb)
BMI (Calculated): 22.5 |
| --- | --- | --- |
Cardiac Rhythm: Normal sinus rhythm
In: 1450 [P.O.:1440; I.V.:10]
Out: 1000 [Urine:1000]

Intake/Output Summary (Last 24 hours) at 1/24/2024 1303
Last data filed at 1/24/2024 0820
[table#9]:
Gross per 24 hour
Intake	1570 ml
Output	1000 ml
Net	**570 ml**

[table#10]:

Last 5 weights
	01/12/24 1931	01/13/24 0205	01/22/24 0411
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)	65.3 kg (144 lb)

EXAM:
GEN: awake, in mld distress
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist mucous membranes, oral ulcers
HEART: RRR, no murmur
LUNGS: lungs clear to auscultation but diminished posterior right lower lobe
ABD: soft, NT, ND,
EXT/MSK: no LE edema, ulnar deviation of bilateral hands and feet,
NEURO: awake and alert, moving all 4 ext to command
Skin/joints: Swan-neck deformity of multiple fingers and toes bilaterally

**Laboratory Data: ** Personally Reviewed

**Imaging: ** Personally Reviewed

Chest x-rays reviewed over several days, notable increasing loculated pleural effusion on right side that has progressed since interval removal of chest tube

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure
2. COPD
3. Right sided empyema, improving
4. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes
5. Heart failure, reduced EF

Recommendations:
- bedside ultrasound reveals a small pocket of fluid that will be amenable to thoracentesis, we will plan for this tomorrow as patient is currently receiving lovenox. Plan to withhold lovenox 1/25AM followed by thoracentesis
- No need for surgical decortication as the lung re-expanded, she is clinically improved and has HFrEF
- Continue Augmentin to complete a total of 4 weeks of antibiotic therapy

Ikramah Latif, DO, 1/24/2024, 1:03 PM
Pulmonary and Critical Care Medicine
#8086509

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system. Patient's risk is high.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dicta
tion system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Cosigned by Christopher Jones, MD at 01/24/2024 1:24 PM PST
Electronically signed by Ikramah Latif, DO at 01/24/2024 1:10 PM PST
Electronically signed by Christopher Jones, MD at 01/24/2024 1:24 PM PST

Associated attestation - Christopher Jones, MD - 01/24/2024 1:24 PM PST
Formatting of this note might be different from the original.
Patient was seen, evaluated, and examined independently by the Resident.

I have also personally performed a face-to-face diagnostic evaluation on this patient. I have reviewed and agree with the care plan documented by the Resident except as noted below.

My plan is as follows: See thoracic ultrasound note, plan for thoracentesis tomorrow, discussed with patient and she is amenable to proceeding. Will hold Lovenox. Continue Augmentin

Plan to restart Advair closer to discharge, has as needed inhalers and has been routinely requesting

Christopher W Jones, MD
Pulmonary and Critical Care Attending

Total time evaluating patient, speaking with patient/family, reviewing records, laboratory data, creating and communicating treatment plan, excluding procedures: 38 minutes

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a moderate complexity case affecting the respiratory system.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Jill F - 01/24/2024 8:54 AM PST
Formatting of this note might be different from the original.

[table#1]:
	01/24/24 0820
Oxygen Therapy/Pulse Ox
Oxygen Therapy	Supplemental oxygen
O2 Delivery Method	Nasal cannula
O2 Flow Rate (L/min)	(S) 0.5 L/min
(On 1 L/min, SpO2=96%. Decreased to 0.5 L/min.)
SpO2	95 %
Home Oxygen	N/A
Patient Activity	At rest
Skin Assessment
Skin Assessment	WDL
Vital Signs
Heart Rate	95
Resp	20
Patient Position	Sitting
Respiratory Assessment
Respiratory Pattern	Normal
Chest Assessment	Symmetrical
Cough	None
R Upper Breath Sounds	Clear
R Lower Breath Sounds	Diminished
L Upper Breath Sounds	Clear
L Lower Breath Sounds	Diminished
Cough/Sputum Description
Sputum Amount	None

Electronically signed by Jill F at 01/24/2024 8:54 AM PST

Omar Khayyat, MD - 01/24/2024 7:45 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on

supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics, she was then started receiving tPA's and dornase through her chest tube for remaining loculations.

Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

1/23/2023
Patient is complaining of neck pain with movement, mostly over the R side but both are painful, this is chronic and has been ongoing for many years, she believes it is headache vs migraine although the patient was able to clearly trace the pain towards the mm of the neck.

1/24
NEON
Patient's foot pain is slightly improving, colchicine dose decreased.
Patient is medically ready to be dced to SNF, unfortunately her insurance is not being accepted by SNFs.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 109/84 (BP Location: Left arm, Patient Position: Lying) | Pulse 82 | Temp 36.8 °C (98.2 °F) (Oral) | Resp 20 | Ht 1.702 m (5' 7") | Wt 65.3 kg (144 lb) | LMP (LMP Unknown) | SpO2 96% | Breastfeeding No | BMI 22.55 kg/m²
I/O: In: 1450 [P.O.:1440; I.V.:10]
Out: 1000 [Urine:1000]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. **R chest tube removed. **
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-**Mild swelling and tenderness of the right foot and deformity of the toes**
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic\
-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/24/24
0548	01/23/24
0544
WBC	K/mcL	4.8	4.2
HGB	g/dL	**8.0***	**7.8***
MCV	fL	**102***	**102***
PLT	K/mcL	364	353
SODIUM	mmol/L	**132***	**128***
POTASSIUM	mmol/L	4.9	4.1
CHLORIDE	mmol/L	98	**96***
CO2	mmol/L	**37***	**37***
BUN	mg/dL	16	16
CREATININE	mg/dL	0.70	0.80
CALCIUM	mg/dL	**8.0***	**7.8***
GLUCOSE	mg/dL	94	92

ALBUMIN	g/dL	**2.6***	**2.5***
AST	Units/L	25	25
ALT	Units/L	17	14
BILIRUBIN TOTAL	mg/dL	0.2	0.2
ALK PHOS	Units/L	74	68

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch

| Result | Value |
| | Strep Pneumo Ag,UR | Positive (A) |
| **Urine culture** |
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus

No results found.

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of br
eath had been started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. Pleural fluid was found to be empyema and patient was intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. Chest tube removed on 1/21/2024.

 # Acute hypoxic respiratory failure
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
 # Bronchial Asthma
 # Possible COPD
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
- Wean supplemental oxygen as tolerated
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- s/p Rocephin and Flagyl (1/12-1/21), transition to** Augmentin x 4 weeks.**
- PRN Duonebs

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt. Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic.
-Follow up regarding anticoagulation outpatient.

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
- Holding Coreg and lisinopril given soft BP
- When appropriate initiate low-dose carvedilol.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

 # AKI - resolved
-Creatinine 1.2 OA, from baseline of 0.9.
- Continue to avoid nephrotoxic medications

**# Hyponatremia, likely SIADH d/t lung pathology-worsening **
Fluid restriction to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265.
- Continue to monitor serum sodium.

 # Right ankle/foot pain, likely gout
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Tapped the joint with insufficient aspirate for culture. Recommends continuation of colchicine treatment .

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray. Lesions have shown much improvement.
- Continue IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.
- Continue oral clotrimazole

#Disposition-
Patient has been evaluated by PT/OT recommended discharge to SNF.

Khayyat, Omar Salim Peter, MD

Electronically signed by Omar Khayyat, MD at 01/25/2024 12:14 AM PST

Jonathan M - 01/23/2024 9:42 PM PST
Formatting of this note might be different from the original.

[table#1]:
	01/23/24 2142
Oxygen Therapy/Pulse Ox
Oxygen Therapy	Supplemental oxygen
O2 Delivery Method	Nasal cannula
O2 Flow Rate (L/min)	(S) 0.5 L/min
(titrated from 1L)
SpO2	98 %
Patient Activity	At rest
$ Pulse Oximetry Spot Check	Yes
**Vital Signs
**
Heart Rate	99
Heart Rate Source	Monitor
Resp	21
Respiratory Assessment
Respiratory Pattern	Normal
Chest Assessment	Chest expansion symmetrical;Symmetrical
Cough	None
R Upper Breath Sounds	Clear
R Lower Breath Sounds	Diminished
L Upper Breath Sounds	Clear
L Lower Breath Sounds	Diminished

Pt is tolerating O2 titration.

Electronically signed by Jonathan M at 01/23/2024 11:06 PM PST

Howie Que, DPM - 01/23/2024 2:09 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was

insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• acyclovir (ZOVIRAX) suspension 400 mg, 400 mg, oral, Q8H SCH, Khoo, Jennifer, PharmD, 400 mg at 01/23/24 0448
• amoxicillin-clavulanate (AUGMENTIN) 875-125 mg per tablet 1 tablet, 1 tablet, oral, BID, Dao, Diep Ngoc, DO, 1 tablet at 01/23/24 0816
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/22/24 0846
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/23/24 0815 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Chen, Yu-Chun, DO, 500 mg at 01/22/24 1001
• clotrimazole (MYCELEX) troche 10 mg, 10 mg, mucous membrane, TID, Dao, Diep Ngoc, DO, 10 mg at 01/23/24 0816
• colchicine (COLCRYS) tablet 1.2 mg, 1.2 mg, oral, Daily, Dao, Diep Ngoc, DO, 1.2 mg at 01/23/24 0816
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Dammert Coronado
, Pedro Manuel, MD, 15 mL at 01/23/24 1204
• enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/23/24 0815
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD, 0.5 mg at 01/23/24 1137
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Chen, Yu-Chun, DO, 25 mg at 01/23/24 0021
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/23/24 1054
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/21/24 1428
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/23/24 0439
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Piatetsky, Yana, MD, 1 patch at 01/23/24 0002
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/16/24 2145
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/13/24 0030 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/23/24 0816
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/23/24 1055
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/23/24 0154
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/22/24 2110

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
| **Vitals:** |

| --- |
	01/23/24 1217
BP:	124/74
Pulse:	85
Resp:	17
Temp:	
SpO2:	98%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
Lower extremity focused physical exam:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may
have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	01/23/24
0544
SODIUM	mmol/L	**128***
POTASSIUM	mmol/L	4.1
CHLORIDE	mmol/L	**96***
CO2	mmol/L	**37***
BUN	mg/dL	16
CREATININE	mg/dL	0.80
GLUCOSE	mg/dL	92
CALCIUM	mg/dL	**7.8***

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)

Howie Que, DPM
01/23/242:10 PM

Electronically signed by Howie Que, DPM at 01/23/2024 2:10 PM PST

Omar Khayyat, MD - 01/23/2024 7:47 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics, she was then started receiving tPA's and dornase through her chest tube for remaining loculations.

Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. Chest tube removed 1/21/24

1/23/2023
Patient is complaining of neck pain with movement, mostly over the R side but both are painful, this is chronic and has been ongoing for many years, she believes it is headache vs migraine although the patient was able to clearly trace the pain towards the mm of the neck.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 102/62 (BP Location: Right arm, Patient Position: Lying) | Pulse 87 | Temp 36.7 °C (98.1 °F) (Oral) | Resp 20 | Ht 1.702 m (5' 7") | Wt 65.3 kg (144 lb) | LMP (LMP Unknown) | SpO2 95% | Breastfeeding No | BMI 22.55 kg/m²
I/O: In: 1528 [P.O.:1508; I.V.:20]
Out: 500 [Urine:500]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. **R chest tube removed. **
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-**Mild swelling and tenderness of the right foot and deformity of the toes**
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

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Data
I have reviewed all of the labs from today. Pertinent labs include:

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0531
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MCV	fL	**102***	100
PLT	K/
mcL	353	368
SODIUM	mmol/L	**128***	**131***

POTASSIUM	mmol/L	4.1	4.2
CHLORIDE	mmol/L	**96***	**96***
CO2	mmol/L	**37***	**35***
BUN	mg/dL	16	15
CREATININE	mg/dL	0.80	0.86
CALCIUM	mg/dL	**7.8***	**7.9***
GLUCOSE	mg/dL	92	105
ALBUMIN	g/dL	**2.5***	**2.4***
AST	Units/L	25	23
ALT	Units/L	14	11
BILIRUBIN TOTAL	mg/dL	0.2	0.2
ALK PHOS	Units/L	68	61

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
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	Influenza B PCR	Not Detected
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	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum

Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus

X-ray portable chest single frontal view

Result Date: 1/22/2024
IMPRESSION: 1. Right chest tube removal with no pneumothorax. There is stable pleural-parenchymal opacity at the right lung base. 2. Mild left basilar atelectatic versus low-grade inflammatory change. release CC Dr: p: f:

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath had been started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray rev
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 # Acute hypoxic respiratory failure
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
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- Wean supplemental oxygen as tolerated
- Patient followed by pulmonology.
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- s/p Rocephin and Flagyl (1/12-1/21), transition to** Augmentin x 4 weeks.**
- PRN Duonebs

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt. Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic.
-Follow up regarding anticoagulation

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
- Holding Coreg and lisinopril given soft BP
- When appropriate initiate low-dose carvedilol.

 # Housing instability
 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up

with the homelessness response center

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-Creatinine 1.2 OA, from baseline of 0.9.
- Continue to avoid nephrotoxic medications

**# Hyponatremia, likely SIADH d/t lung pathology-worsening **
Fluid restriction to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265.
- Continue to monitor serum sodium.

 # Right ankle/foot pain, likely gout
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Tapped the joint with insufficient aspirate for culture. Recommends continuation of colchicine treatment .

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray. Lesions have shown much improvement.
- Continue IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.
- Continue oral clotrimazole

#Disposition-
Patient has been evaluated by PT/OT recommended discharge to SNF.

Khayyat, Omar Salim Peter, MD

Electronically signed by Omar Khayyat, MD at 01/24/2024 7:45 AM PST

Sara T - 01/22/2024 10:08 PM PST
Formatting of this note might be different from the original.

[table#1]:
	01/22/24 2204
Oxygen Therapy/Pulse Ox
Oxygen Therapy	Supplemental oxygen
O2 Delivery Method	Nasal cannula
O2 Flow Rate (L/min)	2 L/min
SpO2	97 %
Patient Activity	At rest
$ Pulse Oximetry Spot Check	Yes
Skin Assessment
Skin Assessment	WDL
Vital Signs
Heart Rate	93
Heart Rate Source	Monitor
Resp	16
Patient Position	Lying
*
*Respiratory Assessment**
Respiratory Pattern	Normal
Chest Assessment	Chest expansion symmetrical;Symmetrical
Cough	None
R Upper Breath Sounds	Clear
R Lower Breath Sounds	Clear
L Upper Breath Sounds	Clear
L Lower Breath Sounds	Clear

Found patient on room air. Nasal cannula placed back on

Electronically signed by Sara T at 01/22/2024 10:08 PM PST

Howie Que, DPM - 01/22/2024 4:20 PM PST
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Typically lives at home with family. Patient notes significantly improved pain in the affected extremity. Ankle tap was insufficient for fluid. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• acyclovir (ZOVIRAX) suspension 400 mg, 400 mg, oral, Q8H SCH, Khoo, Jennifer, PharmD, 400 mg at 01/22/24 1510
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/22/24 0846
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/22/24 0847 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Chen, Yu-Chun, DO, 500 mg at 01/22/24 1001
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Dammert Coronado, Pedro Manuel, MD, 2 g at 01/21/24 2133
• clotrimazole (MYCELEX) troche 10 mg, 10 mg, mucous membrane, TID, Ani, Samuel, MD, 10 mg at 01/22/24 1510
• colchicine (COLCRYS) tablet 1.2 mg, 1.2 mg, oral, Daily, Dao, Diep Ngoc, DO, 1.2 mg at 01/22/24 0846
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Dammert Coronado, Pedro Manuel, MD, 15 mL at 01/22/24 1123
• enoxaparin (LOVENO
X) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/22/24 0847
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD, 0.5 mg at 01/21/24 0804
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Chen, Yu-Chun, DO, 25 mg at 01/21/24 1520
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/22/24 1511
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/21/24 1428
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/22/24 0544
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/16/24 2145
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/13/24 0030 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• metroNIDAZOLE in NaCl (iso-os) (FLAGYL) IVPB 500 mg, 500 mg, intraVENOUS, Q12H, Dammert Coronado, Pedro Manuel, MD,

Last Rate: 100 mL/hr at 01/22/24 1004, 500 mg at 01/22/24 1004
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/22/24 0847
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/22/24 0603
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/21/24 2246
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/21/24 2105

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:
Vitals:
	01/22/24 1555
BP:	119/89
Pulse:	94
Resp:	17
Temp:	36.6 °C (97.9 °F)
SpO2:	95%

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
Lower extremity focused physical exam:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient m
ay have acute gout attack in the ankle at this time. Continue Colchicine therapy.
[table#2]:
Results from last 7 days
Lab	Units	01/22/24
0531
SODIUM	mmol/L	**131***
POTASSIUM	mmol/L	4.2
CHLORIDE	mmol/L	**96***
CO2	mmol/L	**35***
BUN	mg/dL	15
CREATININE	mg/dL	0.86
GLUCOSE	mg/dL	105
CALCIUM	mg/dL	**7.9***

Problem List
)Principal Problem:

Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)

Howie Que, DPM
01/22/244:21 PM

Electronically signed by Howie Que, DPM at 01/22/2024 4:23 PM PST

Your Scripps Health Team - 01/22/2024 12:34 PM PST
Formatting of this note might be different from the original.
**Clinical Nutrition Monitor Note **

Recommendations to Provider:
1. Continue Regular diet, 1500 mL FR per MD
2. Continue daily MVT with minerals

**Nutrition Diagnosis (PES statement): **
Inadequate oral Intake* *related to previous mechanical ventilation, now extubated with PO diet as evidenced by PO intake 69% x 6 charted meals, now meeting estimated kcal needs ~ *revised/improving*

[table#1]:
|
|
| --- |
| [table#1.1] |

[table#1.1]:
Dietary Orders
(From admission, onward)

[table#2]:
|
|
| --- |

[table#3]:
| | [table#3.1]
[table#3.2] |
| --- | --- |

[table#3.1]:
	Start			Ordered
	01/22/24 1101		[table#3.1.1]	01/22/24 1100

[table#3.1.1]:
Combination Diet Regular; 1500 mL; Dental Soft Diet effective now
[table#3.1.1.1]

[table#3.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Fluid Restriction - Oral (mL/day):	1500 mL	
Food Consistency:	Dental Soft	

[table#3.2]:

[table#4]:

Food Allergies: Reviewed (lactose intolerance)

Nutritionally significant labs:
[table#5]:
Results from last 7 days
Lab	Units	01/22/24
0531	01/21/24
1551	01/21/24
0528
SODIUM	mmol/L	**131***	**126***	**127***
,
[table#6]:
Results from last 7 days
Lab	Units	01/22/24
0531	01/21/24
0528	01/20/24
0535
CALCIUM	mg/dL	**7.9***	**8.1***	**8.3***

Nutritionally significant medications:
- enoxaparin, 40 mg, subcutaneous, Daily
- [Held by provider] furosemide, 20 mg, oral, Daily
- multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
- sennosides-docusate sodium, 1 tablet, oral, BID

Skin: Intact

**Anthropometrics: **
Height: 170.2 cm (5' 7"), Weight: 65.3 kg (144 lb), BMI (Calculated): 22.5

Weight trend this admission: [ ] Loss[ ] Gain [x] Stable [ ] Requested new weight
Weight change assessment:
[table#7]:
	Date			
	Height	Weight	BMI (Calculated)	Weight Method
1/12/2024	170.2 cm	—	22.5 kg/m2	
		65.1		
1/13/2024		63.6	22 kg/m2	Bed Scale
1/22/2024		65.318	22.5 kg/m2	

**Estimated needs: **
Weight Used for Equation Calculations: 63.6 kg (140 lb 3.4 oz), Describe Weight Used: Actual
Total Daily Calorie Estimated Needs: 1600-1900 (25-30 kcal/kg) kcal/day
Total Daily Protein Estimated Needs : 65-75 (1-1.2 g/kg) g/day
Total Daily Fluid Estimated Needs: 1600-1900 mL/day

Nutrition Related Summary since last visit:

- Patient seen at bedside; denies any n/v, reports her diarrhea has improved
- Reports she has a great appetite and has been eating well; issues with lactose free diet restriction resolved
- PO intake 84% x 7
charted meals
- Review of meals over past 3 days show average intake of 3153 kcals and 146 grams of protein per day
- Patient is ordering her own meals
- Hyponatremia noted
- Last BM 1/21; no diarrhea
- Reasonable to downgrade to LC2 with continued good PO intake, meeting estimated needs, and stable weight trend

**Is patient meeting estimating needs?: **[x] Yes [ ] No
Are current nutrition related orders adequate and appropriate?: [x] Yes [ ] No
Change recommended: N/A

**Plan of Care Discussion: **Patient and RN

Nutrition Level of Care: Moderate (LC2)

Monitoring/Evaluation:
PO intake, weight trend, BM

Goals:
Patient has a source of nutrition by next RD visit ~ *met*
PO intake to meet GT 75% estimated needs ~ *met/ongoing*

Dakota F., MS, RD
Clinical Nutrition
312-7176

Electronically signed by Your Scripps Health Team at 01/22/2024 12:42 PM PST

Ikramah Latif, DO - 01/22/2024 11:15 AM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista

Chief complain:
Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

History of Present Illness:
This is hospital day 10 for Elizabeth Tina Petruccio a 59 y/o woman admitted with an empyema

Subjective:
No acute overnight events, denies SOB/dyspnea
On 2 liters nasal cannula satting 99%

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
Scheduled Meds:acyclovir, 400 mg, oral, Q8H SCH
cefTRIAXone, 2 g, intraVENOUS, Q24H
clotrimazole, 10 mg, mucous membrane, TID
colchicine, 1.2 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
enoxaparin, 40 mg, subcutaneous, Daily
[Held by provider] furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate, oral, TID PRN, Dao, Diep Ngoc, DO
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate, oral, BID PRN, Chen, Yu-Chun, DO
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• hydrOXYzine pamoate, oral, TID PRN, Chen, Yu-Chun, DO
• ipratropium-albuteroL, nebulization, Q4H PRN, Pena, Jose Rafael, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO

• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Allergies on file: Lactose intolerant [lactose]

Review of Systems:
10 organ review of systems was negative other than what mentioned in HPI

**Family History: **
[table#2]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Fath
er	

Social History:
[table#3]:
Social History

[table#4]:
Socioeconomic History
•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	Not on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance and Sexual Activity
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#5]:
Social Determinants of Health

[table#6]:
Financial Resource Strain: Not on file
Food Insecurity: At Risk (1/19/2024)
	Food Insecurity
	•	In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Yes
Transportation Needs: At Risk (1/19/2024)
	Transportation Needs
	•	In the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Yes
Physical Activity: Not on file
Stress: Not on file
Social Connections: Not on file
Intimate Partner Violence: Not on file
Housing Stability: High Risk (1/19/2024)
	Housing Stability
	•	What is your living situation today?: Do not have a stable place to live

Physical Exam:
[table#7]:
Vitals:
	01/22/24 1000
BP:	
Pulse:	96
Resp:	20
Temp:	
SpO2:	93%

[table#8]:
| Temp: [36.8 °C (98.2 °F)-37.3 °C (99.1 °F)] 36.9 °C (98.4 °F)
Heart Rate: [73-121] 96
Resp: [13-34] 20
BP: (95-115)/(59-78) 100/78
FiO2 (%): [24 %-32 %] 28 % | SpO2: 93 %
FiO2 (%): 28 %
O2 Flow Rate (L/min): 2 L/min | Height: 170.2 cm (5' 7")
Weight: 65.3 kg (144 lb)
BMI (Calculated): 22.5 |
| --- | --- | --- |
Cardiac Rhythm: Normal sinus rhythm
In: 936 [P.O.:810; I.V.:21; IV Piggyback:105]
Out: 850 [Urine:850]

Intake/Output Summary (Last 24 hours) at 1/22/2024 1115
Last data filed at 1/22/2024 0600
[table#9]:
Gross per 24 hour
Intake	715 ml
Output	850 ml
Net	**-135 ml**

[table#10]:
Last 5 weights
	01/12/24 1931	01/13/24 0205	01/22/24 0411
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)	65.3 kg (144 lb)

FiO2 (%): [24 %-32 %] 28 %

EXAM:
GEN: awake, in mld distress
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist mucous membranes, oral ulcers
HEART: RRR, no murmur
LUNGS: diminished in the R base, chest tube in place without erythema
ABD: soft, NT, ND,
EXT/MSK: no LE edema,
NEURO: awake and alert, moving all 4 ext to command

**Laboratory Data: ** Personally Reviewed

**Imaging: ** Personally Reviewed

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure
2. COPD
3. Right sided empyema, improving
4. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes
5. Heart failure, reduced EF

Recommendations:

- Fever, leukocytosis have resolved
- Chest tube removed yesterday. There is a small residual pocket of fluid which is likely to improve and resolve on its own.
- Follow up with repeat cxr
- No need for surgical decortication as the lung re-expanded, she is clinically improved and has HFrEF
- Continue cefriaxone/flagyl - can transition to Augmentin to complete a total of 4 weeks

Ikramah Latif, DO, 1/22/2024, 11:15 AM
Pulmonary and Critical Care Medicine
#8086509

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interva
l during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system. Patient's risk is high.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Cosigned by Pedro Dammert Coronado, MD at 01/23/2024 9:03 AM PST
Electronically signed by Ikramah Latif, DO at 01/22/2024 11:20 AM PST
Electronically signed by Pedro Dammert Coronado, MD at 01/23/2024 9:03 AM PST

Associated attestation - Pedro Dammert Coronado, MD - 01/23/2024 9:03 AM PST
Formatting of this note is different from the original.
I saw and evaluated the patient, participating in the key portions of the service. I reviewed the Resident's note. I agree with the Resident's findings and plan.

Clinically improved. CXR shows residual pocket of pleural fluid. Would continue antibiotics and monitor clinically

Pedro Manuel Dammert Coronado, MD, 1/23/2024, 9:03 AM
Pulmonary and Critical Care Medicine
#808659

Your Scripps Health Team - 01/22/2024 8:48 AM PST
Formatting of this note might be different from the original.
PT/OT rec SNF (walked 10').

Pre Admission Screening and Resident Review (PASRR) completed per the California DHCS (Department of Health Care Services) guidelines. No indication for level II screen. Patient's level I screen will be submitted to accepting SNF per protocol. **CID: 203-348-670**

Spk w/ pt and she agreed to have SNF referrals sent to all facilities in SD County as she verbalized understanding that her straight Mcal might be a barrier. SNF referrals sent.

16:12: No accepting SNF at this time. Spk w/ pt at bedside, and she agreed to have CM place a PRS referral for possible managed Mcal (see below). With pt's permission, SNF referrals re-sent to all SNFs in the county.
[table#1]:
	01/22/24 1628
PRS Referral
Needs PRS Assistance?	Y
Reason for Referral	Urgent, D/C needs causing delay in D/C
Need for Referral	Medi-Cal Changes/Updates;SSA/SSI Questions
Has the need for a PRS referral been communicated to patient?	Yes
Is patient able to manage financial affairs	Yes
Expected date of Discharge	1/24/2024
Additional Communication	Pt has straight Mcal and is interested in possible managed mCal for short term SNF placement; she also wants someone to f/u on her SSI (which she says she applied back in Nov)
Name of CM/SW	Katherine N., RN-CM
Service	Care Manager

BARRIER: underfunded; pt has straight mcal

Electronically signed by Your Scripps Health Team at 01/22/2024 4:39 PM PST

Samuel Ani, DO - 01/22/2024 6:54 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings of residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics, she was then started receiving tPA's and dornase through her chest tube for remaining loculations.

Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. She had a trial of room air, however desatted to 93% after an hour and was placed back on 1 L nasal cannula. Chest tube continues draining.

***01/21/24:** *Chest tube removed per pulmonology, patient states she feels well, however does complain of the feeling of dyspnea despite satting well on nasal cannula. Patient thinks it may be due to her anxiety and asked
whether or not she can receive as needed anxiety meds.

01/22/2024.
No acute events overnight. chest tube removal 01/21/2024. Patient continues to be on 2 L of supplemental oxygen satting at 100%. Patient has been afebrile in the past 48 hours and has had normal WBC in the past 3 days. Net +86 mL in the past 24 hours.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 100/78 (BP Location: Left arm, Patient Position: Lying) | Pulse 89 | Temp 36.9 °C (98.4 °F) (Oral) | Resp 20 | Ht 1.702 m (5' 7") | Wt 65.3 kg (144 lb) | LMP (LMP Unknown) | SpO2 97% | Breastfeeding No | BMI 22.55 kg/m²
I/O: In: 936 [P.O.:810; I.V.:21; IV Piggyback:105]
Out: 850 [Urine:850]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. **R chest tube removed. **
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-**Mild swelling and tenderness of the right foot and deformity of the toes**
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days

| Lab | Units | 01/22/24
0531 | 01/21/24
1551 | 01/21/24
0528 |
WBC	K/mcL	4.8	--	5.7
HGB	g/dL	**8.4***	--	**8.6***
MCV	fL	100	--	100
PLT	K/mcL	368	--	404
SODIUM	mmol/L	**131***	**126***	**127***
POTASSIUM	mmol/L	4.2	--	4.3
CHLORIDE	mmol/L	**96***	--	**96***
CO2	mmol/L	**35***	--	**36***
BUN	mg/dL	15	--	10
CREATININE	mg/dL	0.86	--	0.70
CALCIUM	mg/dL	**7.9***	--	**8.1***
GLUCOSE	mg/dL	105	--	**129***
ALBUMIN	g/dL	**2.4***	--	**2.4***
AST	Units/L	23	--	21
ALT	Units/L	11	--	16
BILIRUBIN TOTAL	mg/dL	0.2	--	0.2
ALK PHOS	Units/L	61	--	57

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen b
y fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date

| **Wound/Fluid/Tissue Culture with stain** |
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus

No results found.

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath had been started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. Pleural fluid was found to be empyema and patient was intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. Chest tube removed on 1/21/2024.

 # Acute hypoxic respiratory failure
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/21
 # S/p extubation 1/14/24
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
- Wean supplemental oxygen as tolerated
- Patient followed by pulmonology, Dr. Dammert, appreciate recs
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- Continue Rocephin and Flagyl (1/12-), can transition to** Augmentin x 4 weeks **when ready for discharge
- PRN Duonebs

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt. Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic.
-Telemetry in the past 48 hours has been nil A-fib/a flutter.
- Recommend outpatient follow-up to discuss anticoagulant

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
- Holding Coreg and lisinopril given soft BP
- When appropriate initiate low-dose carvedilol.

 # Housing instability

 # Substance use disorder (methamphetamine)
-Patient has not had a stable home in the past 2 months.
-Patient had been squatting with friends up to the day of her admission.
-Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

 # AKI - resolved
-Creatinine 1.2 OA, from baseline of 0.9.
- Continue to avoid nephrotoxic medications

**# Hyponatremia, likely SIADH d/t lung pathology-worsening **
Fluid restric
tion to 1.5 L daily
-Likely due to decreased intake or losses not related to kidney disease
- Urine sodium of 17, urine osmolality 152, serum osmolality 265.
- Continue to monitor serum sodium.

 # Right ankle/foot pain, likely gout
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA- negative
- Uric acid 2.6 on 1/19/2024
- Evaluated by podiatry, Dr. Que, on 01/20/2024, who agreed with gouty arthritis. Tapped the joint with insufficient aspirate for culture. Recommends continuation of colchicine treatment .

 # Bronchial Asthma
 # Possible COPD
- DuoNebs as needed
- Patient stated that she gets much of relief of wheezing and SOB if she receives duoneb with pulmicort.
- **No Pulmicort **for now given component of steroids and active infection

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray. Lesions have shown much improvement.
- Continue IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.
- Continue oral clotrimazole

#Disposition-
Patient has been evaluated by PT/OT recommended discharge to SNF. Likely discharge in 1 to 3 days, pending availability of an accepting SNF.

 # Family Updates
1/ 22/2024: Daughter Erika updated, daughter stated that she makes decisions for the family regarding the patient's health. She wanted the social worker to help the patient get into San Diego shelter when she is medically stable.

Samuel Ani, DO
Internal medicine resident ** **

Attending: Dr. Dao, Diep Ngoc, DO

Cosigned by Diep Dao, DO at 01/22/2024 10:08 PM PST
Electronically signed by Samuel Ani, DO at 01/22/2024 2:34 PM PST
Electronically signed by Diep Dao, DO at 01/22/2024 10:08 PM PST

Associated attestation - Diep Dao, DO - 01/22/2024 10:08 PM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note above as needed. I agree with and verify the findings and plan

This is a 55-year-old woman with past medical history of drug induced cardiomyopathy w/ HFrEF 25%, methamphetamine use, and homelessness. She initially presented to the hospital on 4/12 for shortness of breath and was admitted for sepsis with acute hypoxic respiratory failure secondary to a right-sided empyema. Patient was initially intubated and admitted to the ICU. She had a chest tube placed and started on broad spectrum antibiotics. Patient has been extubated on 1/14 and transferred out of the ICU. She has remained stable since then. Workup includes negative blood cultures x 2, pleural fluid culture did not grow any organisms. Her strep pneumoniae urine antigen was positive. Patient has improved with chest tube and antibiotics and her chest tube has

been removed on 1/20. Patient has completed 10 days of IV ceftriaxone and Flagyl. Pulmonary service recommends 4 more weeks of oral Augmentin.

During this office patient patient briefly developed an episode of atrial fibrillation with RVR while in the ICU. Her CHA2DS2-VASc score is 2 and episode is likely related to her sepsis. At this time patient is deferring initiating DOAC for stroke prevention. She would like to establish care with an outpatient cardiologist prior to starting.

The patient also has a history of oral herpes. She states she get herpes flares whenever she is sick or stressed. She has been started on oral acyclovir and also has superimposed candidal infection which she has been started on oral clotrimazole. Her oral pain and lesions have improved significantly and patient is now tolerating a regular diet. Will plan for a total of 10-day course of oral acyclovir and clotrimazole.

During hospitalization patient developed right foot pain and swelling which is consistent with gout flare based on her history along with physical exam findings and X-ray findings. Has been evaluated by podiatry and did a joint aspirate however not enough fluid to test for crystals. No signs of fever or worsening sepsis to suggest any joint infection. Patient improved with oral colchicine.

Patient has been evaluated by PT/OT and they are recommending skilled nursing facility. She has Medi-Cal only and currently no accepting SNF's.

Amy M - 01/22/2024 3:33 AM PST

Formatting of this note might be different from the original.

[table#1]:
	01/22/24 0217
Oxygen Therapy/Pulse Ox
Oxygen Therapy	Supplemental oxygen
O2 Delivery Method	Nasal cannula
O2 Flow Rate (L/min)	(S) 2 L/min
(RT found pt on 3L)
FiO2 (%)	28 %
SpO2	100 %
Patient Activity	At rest
Skin Assessment
Skin Assessment	WDL
Vital Signs
Heart Rate	84
Resp	14
Respiratory Assessment
Respiratory Pattern	Normal
Chest Assessment	Symmetrical
Cough	Non-productive
R Upper Breath Sounds	Expiratory wheezes
R Lower Breath Sounds	Diminished
L Upper Breath Sounds	Expiratory wheezes
L Lower Breath Sounds	Diminished

[table#2]:
	01/22/24 0255
Oxygen Therapy/Pulse Ox
Oxygen Therapy	Supplemental oxygen
O2 Delivery Method	Nasal cannula
O2 Flow Rate (L/min)	2 L/min
SpO2	100 %

Electronically signed by Amy M at 01/22/2024 3:33 AM PST

Yu-Chun Chen, DO - 01/21/2024 3:08 PM PST

Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024.

Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU. She remains on supplemental oxygen, chest tube draining.

On 1/15, patient reported pain in her mouth and lesions on the tongue, suspicious for herpes infection, IV acyclovir started. Pulmonology evaluated patient on 1/16/2024 due to CT chest findings o
f residual fluid in right pleural space as well as uptrending WBC, fever, and intermittent A-fib/a flutter with RVR despite remaining on antibiotics, she was then started receiving tPA's and dornase through her chest tube for remaining loculations.

Her WBC normalized on 01/19/2024. She started complaining of right foot/ankle pain, podiatry consulted to evaluate her for acute gout flare with possible plans for aspiration. She had a trial of room air, however desatted to 93% after an hour and was placed back on 1 L nasal cannula. Chest tube continues draining.

***01/21/24:** *Chest tube removed per pulmonology, patient states she feels well, however does complain of the feeling of dyspnea despite satting well on nasal cannula. Patient thinks it may be due to her anxiety and asked whether or not she can receive as needed anxiety meds.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 115/76 (BP Location: Left arm, Patient Position: Lying) | Pulse 93 | Temp 36.9 °C (98.4 °F) (Oral) | Resp 16 | Ht 1.702 m (5' 7") | Wt 63.6 kg (140 lb 3.4 oz) | LMP (LMP Unknown) | SpO2 99% | Breastfeeding No | BMI 21.96 kg/m²
I/O: In: 605 [P.O.:605]
Out: 640 [Urine:400; Chest Tube:240]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic, **oral lesions improved.**
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. **R chest tube removed. **
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-**Mild swelling and tenderness of the right foot and deformity of the toes**
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/21/24
0528	01/20/24
0535
WBC	K/mcL	5.7	7.6
HGB	g/dL	**8.6***	**9.8***
MCV	fL	100	**101***
PLT	K/mcL	404	**466***
SODIUM	mmol/L	**127***	**129***
POTASSIUM	mmol/L	4.3	4.9
CHLORIDE	mmol/L	**96***	**96***
CO2	mmol/L	**36***	**35***
BUN	mg/dL	10	12
CREATININE	mg/dL	0.70	0.66
CALCIUM	mg/dL	**8.1***	**8.3***
GLUCOSE	mg/dL	**129***	96
ALBUMIN	g/dL	**2.4***	**2.5***
AST	Units/L	21	18

ALT	Units/L	16	12
BILIRUBIN TOTAL	mg/dL	0.2	0.3
ALK PHOS	Units/L	57	64

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen
: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value

| | Strep Pneumo Ag,UR | Positive (A) |
| **Urine culture** |
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus

X-ray portable chest single frontal view

Result Date: 1/21/2024
IMPRESSION: Right pleural catheter in place. Small to moderate pleural effusion and patchy right basilar opacity, probably atelectasis. No pneumothorax. release

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath had been started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. Pleural fluid was found to be empyema and patient was intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

 # Acute hypoxic respiratory failure
 # Sepsis secondary to right-sided empyema with chest tube placement on 1/13-1/19
 # S/p extubation 1/14/24
S/p Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance. Strep pneumonia antigen positive 01/14. Chest CT performed 1/16/2024 showed residual right pleural fluid. Flu and COVID-negative
- Wean supplemental oxygen as tolerated
- Patient followed by pulmonology, Dr. Dammert, appreciate recs
- Chest tube removed on 1/21, per pulmonology no need for surgical decortication as the lung reexpanded and patient is clinically improved and has HFrEF
- Continue Rocephin and Flagyl (1/12-), can transition to Augmentin x 4 weeks when ready for discharge
- PRN Duonebs

 # A-fib RVR (CHA2DS2-VASc score of 2)
S/p amnio gtt. Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic. 1/17/2024, night, was negative for A-fib/a flutter
- Consider amiodarone if if A-fib/flutter continues
- Obtain twelve-lead EKG as needed
- Recommend outpatient follow up to discuss anticoagulant

 # HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation.
- Holding Coreg and lisinopril given soft BP
- When appropriate
initiate low-dose carvedilol.

 # Housing instability
 # Substance use disorder (methamphetamine)
Patient has not had a stable home in the past 2 months.
Patient had been squatting with friends up to the day of her admission.
- Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

 # AKI - resolved
Creatinine 1.2 OA, from baseline of 0.9.
- Continue to avoid nephrotoxic medications

 # Hyponatremia, likely SIADH d/t lung pathology

Fluid restriction to 1.5 L daily
- Urine sodium of 17, urine osmolality 152, serum osmolality 265.
- Continue to monitor serum sodium, recheck in the evening

 # Right ankle/foot pain, likely gout
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule out gout
- Right foot x-ray on 1/19/2024 showed chronic erosions of the distal first metatarsal likely related to gout.
- RF, ANA, pending
- Uric acid 2.6 on 1/19/2024
- Podiatry Dr. Que consulted, to evaluate for joint aspiration

 # Bronchial Asthma
 # Possible COPD
- DuoNebs as needed
- Patient stated that she gets much of relief of wheezing and SOB if she receives duoneb with pulmicort.
- No Pulmicort for now given component of steroids and active infection

**# Oral Ulcers, likely Herpes outbreak **
 # Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray.
- Continue IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.
- Continue oral clotrimazole

 # Family Updates
1/19/2024: Daughter Erika updated, daughter stated that she makes decisions for the family regarding the patient's health. She wanted the social worker to help the patient get into San Diego shelter when she is medically stable.

Resident:
Yu-Chun Chen, DO
Internal Medicine, PGY-2

Attending: Dr. Dao, Diep Ngoc, DO

Cosigned by Diep Dao, DO at 01/21/2024 8:13 PM PST
Electronically signed by Yu-Chun Chen, DO at 01/21/2024 3:33 PM PST
Electronically signed by Diep Dao, DO at 01/21/2024 8:13 PM PST

Associated attestation - Diep Dao, DO - 01/21/2024 8:13 PM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note above as needed. I agree with and verify the findings and plan

Your Scripps Health Team - 01/21/2024 9:08 AM PST
Formatting of this note might be different from the original.
Admitted for sob & sepsis 2/2 empyema. S/p chest tube placed 1/13 & removed 1/21. IV abx. S/p intubation 1/13 & extubation 1/14. Hx of homelessness, etoh, & meth use. PT/OT recs snf. Walked 10'. Stays w/ son Anthony in Huntington Beach shelters or travels in MX, indp pta. Dtr Erika lives in NJ. Medi-cal only. Dcp: Evolving
**SHEILA MARIE P. **
CM/RN/BSN
SCRIPPS MERCY CHULA VISTA

Electronically signed by Your Scripps Health Team at 01/21/2024 9:10 AM PST
Electronically signed by Your Scripps Health Team at 01/21/2024 3:39 PM PST

Pedro Dammert Coronado, MD - 01/21/2024 7:21 AM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista

Chief complain:
Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

History of Present Illness:
This is hospital day 9 for Elizabeth Tina Petruccio a 59 y/o woman admitted with an empyema

Subjective:
Feels better, denies SOB

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
Scheduled Meds:acyclovir, 400 mg, oral, Q8H SCH
cefTRIAXone, 2 g, intraVENOUS, Q24H
clotrimazole, 10 mg, mucous membrane, TID
colchicine, 0.6 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
[Held by provider] enoxaparin, 40 mg, subcutaneous, Daily
furosemide, 20 mg, oral, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal
, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate, oral, TID PRN, Dao, Diep Ngoc, DO
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• ipratropium-albuteroL, nebulization, Q4H PRN, Pena, Jose Rafael, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Allergies on file: Lactose intolerant [lactose]

Review of Systems:
10 organ review of systems was negative other than what mentioned in HPI

**Family History: **
[table#2]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Social History:
[table#3]:
Social History

[table#4]:
Socioeconomic History

•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	Not on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance and Sexual Activity
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#5]:
Social Determinants of Health

[table#6]:
Financial Resource Strain: Not on file
Food Insecurity: At Risk (1/19/2024)
	Food Insecurity
	•	In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Yes
Transportation Needs: At Risk (1/19/2024)
	Transportation Needs
	•	In the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Yes
Physical Activity: Not on file
Stress: Not on file
Social Connections: Not on file
Intimate Partner Violence: Not on file
Housing Stability: High Risk (1/19/2024)
	Housing Stability
	•	What is your living situation today?: Do not have a stable place to live

Physical Exam:
[table#7]:
Vitals:
	01/21/24 0400
BP:	112/80
Pulse:	93
Resp:	20
Temp:	37.2 °C (99 °F)
SpO2:	95%

[table#8]:
| Temp: [36.7 °C (98.1 °F)-37.3 °C (99.1 °F)] 37.2 °C (99 °F)
Heart Rate: [93-114] 93
Resp: [16-27] 20
BP: (105-123)/(67-87) 112/80
FiO2 (%): [28 %] 28 % | SpO2: 95 %
FiO2 (%): 28 %
O2 Flow Rate (L/min): 2 L/min | Height: 170.2 cm (5' 7")
Weight: 63.6 kg (140 lb 3.4 oz)
BMI (Calculated): 22 |
| --- | --- | --- |
Cardiac Rhythm: Sinus tachycardia
In: 605 [P.O.:605]
Out: 640 [Urine:400; Chest Tube:240]

Intake/Output Summary (Last 24 hours) at 1/21/2024 0721
Last data filed at 1/21/2024 0641

[table#9]:
Gross per 24 hour
Intake	605 ml

| Output | 640 ml |
| **Net** | **-35 ml** |

[table#10]:
Last 5 weights
	01/12/24 1931	01/13/24 0205
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)

FiO2 (%): [28 %] 28 %

EXAM:
GEN: awake, in mld distress
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist mucous membranes, oral ulcers
HEART: RRR, no murmur
LUNGS: diminished in the R base, chest tube in place without erythema
ABD: soft, NT, ND,
EXT/MSK: no LE edema,
NEURO: awake and alert, moving all 4 ext to command

**Laboratory Data: ** Personally Reviewed

**Imaging: ** Personally Reviewed

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure
2. Right sided empyema
3. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes

Recommendations:
- Fever, leukocytosis and general status have improved significant
- Chest tube removed today. There is a small residual pocket of fluid but should probably resolve on its own as clinically ha improved
- No need for surgical decortication as the lung re-expanded, she is clinically improved and has HFrEF
- Continue cefriaxone/flagyl - can transition to Augmentin to complete a total of 4 weeks

Pedro Manuel Dammert Coronado, MD, 1/21/2024, 7:21 AM
Pulmonary and Critical Care Medicine
#8086509

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system. Patient's risk is high.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Pedro Dammert Coronado, MD at 01/21/2024 7:23 AM PST

Formatting of this note might be different from the original.
Admitted for sob & sepsis 2/2 empyema. S/p chest tube placed 1/13. IV abx. S/p intubation 1/13 & extubation 1/14. Hx of homelessness, etoh, & meth use. OT recs snf. Stays with son Anthony in Huntington Beach shelters or travels in MX, indp pta. Dtr Erika lives in NJ. See SWA 1/15. Medi-cal. Dcp: Evolving
**SHEILA MARIE P. **
CM/RN/BSN
SCRIPPS MERCY CHULA VISTA

Electronically signed by Your Scripps Health Team at 01/20/2024 4:04 PM PST

Pedro Dammert Coronado, MD - 01/20/2024 9:30 AM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista

Chief complain:
Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

History of Present Illness:
This is hospital day 8 for Elizabeth Tina Petruccio a 59 y/o woman admitted with an empyema

Subjective:
Mild SOB
Was agitated overnight

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
Scheduled Meds:acyclovir, 400 mg, oral, Q8H SCH
cefTRIAXone, 2 g, intraVENOUS, Q24H
clotrimazole, 10 mg, mucous membrane, TID
colchicine, 0.6 mg, oral, Daily
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
[Held by provider] enoxaparin, 40 mg, subcutaneous, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• Bowel Regim
en of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• ipratropium-albuteroL, nebulization, Q4H PRN, Pena, Jose Rafael, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Allergies on file: Patient has no known allergies.

Review of Systems:
10 organ review of systems was negative other than what mentioned in HPI

**Family History: **
[table#2]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Social History:
[table#3]:
Social History

[table#4]:
Socioeconomic History
•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	Not on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance and Sexual Activity
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#5]:
Social Determinants of Health

[table#6]:
Financial Resource Strain: Not on file
Food Insecurity: At Risk (1/19/2024)
	Food Insecurity
	•	In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Yes
Transportation Needs: At Risk (1/19/2024)
	Transportation Needs
	•	In the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Yes
Physical Activity: Not on file
Stress: Not on file
Social Connections: Not on file
Intimate Partner Violence: Not on file
Housing Stability: High Risk (1/19/2024)
	Housing Stability
	•	What is your living situation today?: Do not have a stable place to live

Physical Exam:
[table#7]:
Vitals:
	01/20/24 0709
BP:	110/84
Pulse:	97
Resp:	15
Temp:	37.2 °C (99 °F)
SpO2:	99%

[table#8]:
| Temp: [36.5 °C (97.7 °F)-37.2 °C (99 °F)] 37.2 °C (99 °F)
Heart Rate: [97-112] 97
Resp: [15-23] 15
BP: (101-122)/(74-86) 110/84
FiO2 (%): [24 %-28 %] 28 % | SpO2: 99 %
FiO2 (%): 28 %
O2 Flow Rate (L/min): 2 L/min | Height: 170.2 cm (5' 7")
Weight: 63.6 kg (140 lb 3.4 oz)
BMI (Calculated): 22 |
| --- | --- | --- |
Cardiac Rhythm: Sinus tachycardia
In: 920 [P.O.:720; IV Piggyback:200]
Out: 1430 [Urine:600; Chest Tube:830]

Intake/Output Summary (Last 24 hours) at 1/20/2024 0930
Last data filed at 1/20/2024 0847
[table#9]:
Gross per 24 hour
Intake	1040 ml
Output	1430 ml
Net	**-390 ml**

[table#10]:
| **Last 5 weights
** |
| --- |
| | 01/12/24 1931 | 01/13/24 0205 |
| Weight: | 65.1 kg (143 lb 8.3 oz) | 63.6 kg (140 lb 3.4 oz) |

FiO2 (%): [24 %-28 %] 28 %

EXAM:
GEN: awake, in mld distress
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist mucous membranes, oral ulcers
HEART: RRR, no murmur
LUNGS: diminished in the R base, chest tube in place without erythema
ABD: soft, NT, ND,
EXT/MSK: no LE edema,
NEURO: awake and alert, moving all 4 ext to command

**Laboratory Data: ** Personally Reviewed

**Imaging: ** Personally Reviewed

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure
2. Right sided empyema
3. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes

Recommendations:
- Fever, leukocytosis and general status have improved significant
- CXR shows that lung re-expanded except a small basilar pocket
- Keep chest tube to suction
- Plan to remove tube tomorrow 1/21
- No need for surgical decortication as the lung re-expanded well
- Continue cefriaxone/flagyl
- Pain control

Pedro Manuel Dammert Coronado, MD, 1/20/2024, 9:30 AM
Pulmonary and Critical Care Medicine
#8086509

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple

interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system. Patient's risk is high.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Pedro Dammert Coronado, MD at 01/20/2024 9:40 AM PST

Samuel Ani, DO - 01/20/2024 5:26 AM PST

Formatting of this note might be different from the original.

[image]

Hospital Medicine Progress Note

Subjective

[table#1]:

Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

01/15/2024
Patient seen and examined at bedside. Reported pain in the mouth and some lesions on the tongue.
Patient stated that she usually gets the lesion whenever she is sick. Lesion responds to numbing oral spray,
Per patient. Patient continues to require supplemental oxygen improved from 4 L to 2 L today.

01/16/2024
Continues to have pain in the mouth no reported use of some balm provides some relief. Counseled patient against putting in the mouth because of associated chemicals that could have deleterious effects. Reported she will apply to the lips only. Supplemental oxygen 90 continues to improve currently on 1 L from 2 L 1/15/2024. Mild pain in the lower extremity, right. WBC up trended this morning and patient noted to have an episode of fever spike 1/15/24-concerning for persistent infection in spite of antibiotics. CT chest repeated today showing residual fluid in the right pleural space. Evaluated by pulmonology that plans tPA/DNase intrapleural injection for remaining loculations.

01/17/24
Overnight noted to have intermittent shortness of breath and generalized body w
eakness with runs of A-fib/a flutter 110-149 nonsustained asymptomatic. This morning reported improvement of oral soreness. Extremity pain improving with analgesic. Received chest tube flushing as well as tPA/denies intrapleural injection. Much improvement with respect to output 500 mL yesterday and today. Adjustment made to the location of the chest tube. Chest tube site noted to be erythematous. Low urine output (350 mL) in the past 24 hours, with net balance of 1 L. Blood pressure has been soft for which intervention 250 mL of normal saline bolus was made this morning. Patient not a candidate for beta-blockers because of soft blood pressure as well as some recent ejection fraction of less than 30. Will continue to monitor.

01/18/2024
Supplemental oxygen need improved from 2 L NC to 1 L NC. She is net 592.4 mL in the past 24 hours with chest tube drain output of 125 mL. Patient has been afebrile in the past 24 hours. WBC continues to downtrend 11.6 from 12.8. TPA/DNAse on hold by pulmonologist to evaluate the erythema noted at chest tube insertion site. No recorded A-fib/a flutter overnight.

01/19/2024
No acute events overnight. Afebrile. Trial of room air revealed SpO2 of 94% , was put back on 1 L when patient desatted to 93% after about 1 hour. Right foot pain. Continues to need 1 L supplemental oxygen via nasal cannula satting at 100%. She is net +1 L in the last 24 hours, with right chest tube 160 mL also in the last 24 hours, continues to decrease in the last 2 days. WBC normalized to 9.6 from 11.6, first normal value since admission 7 days ago.

1/20/2024
Overnight, patient reported anxiety and difficulty with breathing for which she was medicated with Ativan and breathing treatments performed which were effective. Continues to need 2 L of supplemental oxygen. Net -210 mL in the past 24 hours. Chest tube output of 530 ml in the last 24 hours.

[table#2]:

ROS

A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 123/87 (BP Location: Left arm) | Pulse 97 | Temp 37.2 °C (99 °F) (Oral) | Resp 17 | Ht 1.702 m (5' 7") | Wt 63.6 kg (140 lb 3.4 oz) | LMP (LMP Unknown) | SpO2 98% | Breastfeeding No | BMI 21.96 kg/m^2
I/O: In: 920 [P.O.:720; IV Piggyback:200]
Out: 1430 [Urine:600; Chest Tube:830]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. Right chest tube present and functional.
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-**Mild swelling and tenderness of the right foot and deformity of the toes**
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/20/24
0535	01/19/24
0422	01/15/24
0528	01/14/24
0502	01/14/24
0022	01/13/24
2028
WBC	K/mcL	7.6	9.6	< >	**14.3***	--	--
HGB	g/dL	**9.8***	**10.2***	< >	**9.7***	--	--
MCV	fL	**101***	**102***	< >	97	--	--
PLT	K/mcL	**466***	**426***	< >	**573***	--	--
SODIUM	mmol/L	**129***	**130***	< >	**133***	**134***	**132***
POTASSIUM	mmol/L	4.9	4.7	< >	4.6	4.6	4.7
CHLORIDE	mmol/L	**96***	100	< >	104	104	103
CO2	mmol/L	**35***	**32***	< >	26	28	27
BUN	mg/dL	12	14
< >	**50***	**53***	**56***
CREATININE	mg/dL	0.66	0.60	< >	**1.20***	**1.22***	**1.30***
CALCIUM	mg/dL	**8.3***	**8.2***	< >	**8.1***	**8.3***	**8.0***
MAGNESIUM	mg/dL	--	--	--	2.3	--	--
PHOSPHORUS	mg/dL	--	--	--	--	2.9	3.5
GLUCOSE	mg/dL	96	94	< >	**144***	**173***	**135***
ALBUMIN	g/dL	**2.5***	**2.6***	< >	--	**2.6***	**2.7***
AST	Units/L	18	24	< >	--	--	--
ALT	Units/L	12	13	< >	--	--	--
BILIRUBIN TOTAL	mg/dL	0.3	0.3	< >	--	--	--
ALK PHOS	Units/L	64	62	< >	--	--	--
CK	Units/L	--	--	--	128	--	--
< > = values in this interval not displayed.

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected

| **COVID-19, Rapid (PCR/NAAT COMBO)** |
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.

X-ray portable chest single frontal view STAT

Result Date: 1/20/2024
IMPRESSION: Stable position of right chest tube, some loculated pleural fluid and no pneumothorax. Mild bibasilar volume loss. These findings are concordant with the preliminary interpretation provided by StatRad radiologist, Statrad Code: A release CC Dr: p: f:

X-ray portable chest single frontal view

Result Date: 1/20/2024
IMPRESSION: Right-sided pigtail chest tube remains in place. There is minimal basilar volume loss. C
ardiomegaly again noted. Mild redistribution. release CC Dr: p: f:

X-ray foot right 3 or more views

Result Date: 1/19/2024
IMPRESSION: 1. Chronic erosions of the distal first metatarsal likely related to gout 2.. No acute bony pathology. release

X-ray chest single frontal view

Result Date: 1/19/2024
IMPRESSION: No change. release

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath had been started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. Pleural fluid was found to be empyema and patient was intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

#Hypoxic respiratory failure
#Sepsis secondary to right-sided empyema with chest tube placement on 1/13-improving
Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance.
-Extubated 1/14
Strep pneumonia antigen positive 01/14
Chest CT performed 1/16/2024 showed residual right pleural fluid.
-**Wean supplemental oxygen as tolerated.**
-Normal white count 9.6 on 1/19/2024
-Right chest tube output 160 mL in the last 24 hours, output continues to decrease in the last 48 hours
-Patient followed by pulmonology, Dr. Dammert, appreciate recs
-Pleural fluid culture collected 1/13, no growth to date
-AFB culture with stain performed on 01/13/2024-no AFB stain
-Blood culture, collected 01/12/2024, no growth to date
-Influenza A/B and COVID-19 rapid PCR/NAAT-both negative
-Continue Rocephin and Flagyl (1/12-)

#A-fib RVR (CHA2DS2-VASc score of 2)
-S/p amnio gtt.
Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic. 1/17/2024, night, was negative for A-fib/a flutter
-Consider amiodarone if if A-fib/flutter continues
-Obtain twelve-lead EKG
-Will continue gentle rehydration.
-Recommend outpatient follow up to discuss anticoagulant.

#HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation
-Holding Coreg and lisinopril given soft BP
-When appropriate initiate low-dose carvedilol.

 # Housing instability
#Substance use disorder-methamphetamine
Patient has not had a stable home in the past 2 months.
Patient had been squatting with friends up to the day of her admission
Patient was seen by social worker on 01/19/2024 and was provided with resources for substance use disorder and for follow-up with the homelessness response center

#AKI-resolved
Creatinine 1.2 OA, from baseline of 0.9.
Normal value 1/15/2024
Continue to avoid nephrotoxic medications

#Hyponatremia
Fluid restriction to 1.5 L daily
-Urine sodium and creatinine pending
-Continue to monitor serum sodium

#Right ankle/foot pain
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule out gout
-Right foot x-ray on 1/19/2024 showed Chronic erosions of the distal first metatarsal likely related to gout.
-Right foot x-ray
-RF, ANA, pending
-Uric acid-2.6 on 1/19/2024
-Podiatry consult, Dr. Que, send.

 # Bronchial Asthma
 # Possible COPD.
-DuoNebs as needed
-Patient stated that she gets much of relief of wheezing and SOB if she receives duoneb with pulmicort.
-will add pulmicort PRN

 # Mouth pain and ulcers-much improvement
-Likely oral herpes
-Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examina
tion of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray.
-Continue IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.
-Continue oral clotrimazole

Updated daughter, Erika, 1/19/2024. Erika stated that she makes decisions for the family regarding the patient's health. She wanted the social worker to help the patient get into San diego shelter when she is medically stable.
Samuel Ani, DO
Internal medicine resident ** **

Attending: Dr. Dao, Diep Ngoc, DO

Cosigned by Diep Dao, DO at 01/21/2024 9:58 AM PST
Electronically signed by Samuel Ani, DO at 01/20/2024 3:34 PM PST
Electronically signed by Samuel Ani, DO at 01/20/2024 6:31 PM PST
Electronically signed by Diep Dao, DO at 01/21/2024 9:58 AM PST

Associated attestation - Diep Dao, DO - 01/21/2024 9:58 AM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note above as needed. I agree with and verify the findings and plan

Formatting of this note might be different from the original.
Received RN referral from Erika B re: Housing, homeless, DV and Advance Directive, comment: pt is asking for housing resources in San Diego County, previously lived in Orange County. Pt was seen by SW Ada R this admission and provided w/ homeless resources. Please see prior SW note for full details. Spiritual Care saw pt earlier today and provided her w/ Advance Directive information.

Reviewed medical record and met with pt at bedside to obtain the above information. Pt presents as AAOX4, yet a poor historian, tangential and disjointed. She was perseverating on her housing situation in 2020 and often repeated herself. She felt it was important to note that she had been living in an apartment w/ her son (Anthony) in 2020. She went to stay w/ a friend (Jose Jimenez, unknown phone number) in Rosarito, Mexico and when she returned, her son had lost the apartment. Pt and son then were staying together at the Navigation Center in Huntington Beach,while trying to obtain stable housing. She would periodically go to say w/ her friend, Jose, in Mexico. She feels that her son is having "his own issues" and it she is better off staying away from him. Therefore, she went to stay w/ her friend (Jose) in Mexico in December 2023. She did not return within a week and has been exited from the Huntington Beach Navigation Center program. She must wait 30 days before she can return. At this point, she feels she will stay in San Diego, because she does not want to be around her son when he's "having issues". Pt provided w/ multiple homeless resources, including the Homelessness Response Center, 2-1-1 and inclement weather shelters. Pt does not have funding for ILF.

When she was last in Mexico, she left her friend (Jose's) home because there was no heat. Instead, she went to stay w/another friend (Odie Ramirez Castillo) in Rosarito, Mexico, who also had not heat in his home. Pt states after 2 days, he became angry and physically abusive. Pt stated he punched her in the mouth, punched her in the back and also kicked her in the back. This occurred over a 2-3 day period, until she was able to get "the lady who rents the room upstairs" to unlock the front gait and let her out. Pt does not know the address or the lady's name. It is very difficult to get a clear statement from her. Pt has no injuries and was noted by ED Dr. Zanoni to have no rash or bruising on admission. Discussed w/ RN Cherry L, who notes pt has no injuries or bruising, therefore no Suspicious Injury Report is warranted. I returned to pt and asked if she would like to make a police report. Pt stated that she would. The incident occurred in Rosarito Mexico, therefore, she will need to make the report there. Pt provided w/ the Rosarito Municipal Police (Gral. Jauan Vaicario 225, Benito Juarez, 22710 Playas de Rosarito, B.C., Mexico).

Lastly, I provided pt w/ a list of substance abuse resources in the area, given her history of methamphetamine abuse. Pt denies current use but did accept the resource.

Pt was appreciative of the resources. At this time, she intends to f/up w/ the Homelessness Response Center when medically stable. Weather appropriate clothing noted in pt.'s closet. RN can provide a taxi to the HRC or a bus pass at d/c.

Plan: This department will continue to monitor this case to ensure a safe d/c plan.
Bett
ina S.

Electronically signed by Your Scripps Health Team at 01/19/2024 4:35 PM PST

Sandra S - 01/19/2024 11:16 AM PST
Summary: oxygen rounds

Formatting of this note might be different from the original.

[table#1]:
	01/19/24 1116
Oxygen Therapy/Pulse Ox
Oxygen Therapy	Supplemental oxygen
O2 Delivery Method	Nasal cannula
O2 Flow Rate (L/min)	2 L/min
FiO2 (%)	28 %
SpO2	98 %
Skin Assessment
Skin Assessment	WDL
Vital Signs
Heart Rate	98
Resp	18

Pt did not tolerate 1 lpm nc. Due to anxiety episode. Fio2 was variable and ultimately Fio2 was increased back to 2 lpm nc. Pulse oximetry was changed.

Pt was coach to slow down RR freq. deep breath/ purse lip breathing. Spo2 were 98%

Electronically signed by Sandra S at 01/19/2024 11:19 AM PST

Sandra S - 01/19/2024 10:00 AM PST

Summary: oxygen rounds

Formatting of this note might be different from the original.

[table#1]:
	01/19/24 0959
Oxygen Therapy/Pulse Ox
Oxygen Therapy	(S) Supplemental oxygen
O2 Delivery Method	(S) Nasal cannula
O2 Flow Rate (L/min)	(S) 1 L/min
FiO2 (%)	(S) 24 %
SpO2	100 %
Home Oxygen	(S) N/A
Patient Activity	(S) At rest
$ Pulse Oximetry Spot Check	(S) Yes
Skin Assessment
Skin Assessment	(S) WDL
Vital Signs
Heart Rate	98
Resp	19

Pt was titrated from 2 lpm nc to 1lpm nc

On 2 lpm nc pt Spo2 was 100%.

With titration to 1 lpm . Pt SPO2 is 95%

Sandra M. Sandoval-Medina, RT, 1/19/2024, 10:01 AM

Electronically signed by Sandra S at 01/19/2024 10:01 AM PST

Pedro Dammert Coronado, MD - 01/19/2024 9:47 AM PST

Formatting of this note might be different from the original.

Pulmonary Medicine Attending Progress Note - Scripps Chula Vista

Chief complain:
Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

History of Present Illness:
This is hospital day 7 for Elizabeth Tina Petruccio a 59 y/o woman admitted with an empyema

Subjective:
Less SOB
Looks better, more awake, no distress

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
Scheduled Meds:acyclovir, 400 mg, oral, Q8H SCH
cefTRIAXone, 2 g, intraVENOUS, Q24H
clotrimazole, 10 mg, mucous membrane, TID
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
enoxaparin, 40 mg, subcutaneous, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• ipratropium-albuteroL, nebulization, Q4H PRN, Ho, Truc Viet, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Allergies on file: Patient has no known allergies.

Review of Systems:
10 organ review of systems was negative other than w
hat mentioned in HPI

**Family History: **
[table#2]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Social History:
[table#3]:
Social History

[table#4]:
Socioeconomic History
•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	Not on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance and Sexual Activity
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#5]:
Social Determinants of Health

[table#6]:
Financial Resource Strain: Not on file
Food Insecurity: At Risk (1/19/2024)
	Food Insecurity
	•	In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Yes

| Transportation Needs: At Risk (1/19/2024) |
| | Transportation Needs |
| | • | In the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Yes |
| Physical Activity: Not on file |
| Stress: Not on file |
| Social Connections: Not on file |
| Intimate Partner Violence: Not on file |
| Housing Stability: High Risk (1/19/2024) |
| | Housing Stability |
| | • | What is your living situation today?: Do not have a stable place to live |

Physical Exam:
[table#7]:
Vitals:
	01/19/24 0726
BP:	
Pulse:	104
Resp:	21
Temp:	
SpO2:	92%

[table#8]:
| Temp: [36.6 °C (97.9 °F)-37.5 °C (99.5 °F)] 36.6 °C (97.9 °F)
Heart Rate: [89-108] 104
Resp: [16-23] 21
BP: (103-126)/(73-87) 126/80
FiO2 (%): [21 %-28 %] 21 % | SpO2: 92 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 1 L/min | Height: 170.2 cm (5' 7")
Weight: 63.6 kg (140 lb 3.4 oz)
BMI (Calculated): 22 |
| --- | --- | --- |
Cardiac Rhythm: Normal sinus rhythm
In: 1248.6 [P.O.:480; I.V.:413.6; IV Piggyback:355]
Out: 160 [Chest Tube:160]

Intake/Output Summary (Last 24 hours) at 1/19/2024 0947
Last data filed at 1/19/2024 0524
[table#9]:
Gross per 24 hour
Intake	527.57 ml
Output	160 ml
Net	**367.57 ml**

[table#10]:
Last 5 weights
	01/12/24 1931	01/13/24 0205
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)

FiO2 (%): [21 %-28 %] 21 %

EXAM:
GEN: awake, in mld distress
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist mucous membranes, oral ulcers
HEART: RRR, no murmur
LUNGS: diminished in the R base, chest tube in place without erythema
ABD: soft, NT, ND,
EXT/MSK: no LE edema,
NEURO: awake and alert, moving all 4 ext to command

**Laboratory Data: ** Personally Reviewed

**Imaging: ** Personally Reviewed

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure
2. Right sided empyema
3. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes

Recommendations:
- Fever, leukocytosis and general status have improved significant
- CXR shows that lung re-expanded except a small basilar pocket
- Will give one dose of TPA/DNAse today
- Keep chest tube to suction
- Plan to remove tube in 1-2 days
- No need for surgical decortication as the lung re-expanded well
- Continue cefriaxone/flagyl
- Pain control

Pedro Manuel Dammert Coronado, MD, 1/19/2024, 9:47 AM
Pulmonary and Critical Care Medicine
#8086509

E&M coding
I have seen and examined the patient today and by signing
above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system. Patient's risk is high.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Pedro Dammert Coronado, MD at 01/19/2024 9:49 AM PST

Your Scripps Health Team - 01/19/2024 9:38 AM PST
Formatting of this note might be different from the original.
**Clinical Nutrition Monitor Note **

Recommendations to Provider:
1. Continue Regular diet, Dysphagia 3 per SLP
2. Continue daily MVT with minerals

**Nutrition Diagnosis (PES statement): **
Inadequate oral Intake* *related to previous mechanical ventilation, now extubated with PO diet as evidenced by PO intake 69% x 6 charted meals, now meeting estimated kcal needs ~ *revised/improving*

[table#1]:
|
|
| --- |
| [table#1.1] |

[table#1.1]:
Dietary Orders
(From admission, onward)

[table#2]:
|
|
| --- |

[table#3]:
| | [table#3.1]
[table#3.2] |
| --- | --- |

[table#3.1]:

	Start			Ordered
	01/15/24 1048		[table#3.1.1]	01/15/24 1047

[table#3.1.1]:
Combination Diet Regular; Dysphagia 3 Advanced Texture Diet effective now
[table#3.1.1.1]

[table#3.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Food Consistency:	Dysphagia 3 Advanced Texture	

[table#3.2]:

[table#4]:

Food Allergies: NKFA

Nutritionally significant labs:
[table#5]:
Results from last 7 days
Lab	Units	01/19/24
0422	01/18/24
0519	01/17/24
0519
SODIUM	mmol/L	**130***	**132***	**132***
,
[table#6]:
Results from last 7 days
Lab	Units	01/19/24
0422	01/18/24
0519	01/17/24
0519
CALCIUM	mg/dL	**8.2***	**8.3***	8.6

Nutritionally significant medications:
- enoxaparin, 40 mg, subcutaneous, Daily
- metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
- multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
- sennosides-docusate sodium, 1 tablet, oral, BID (not given)

Skin: Intact

**Anthropometrics: **
Height: 170.2 cm (5' 7"), Weight: 63.6 kg (140 lb 3.4 oz), BMI (Calculated): 22

Weight trend this admission: [ ] Loss[ ] Gain [ ] Stable [x] Requested new weight
Weight change assessment:
[table#7]:
	Date			
	Height	Weight	BMI (Calculated)	Weight Method
1/12/2024	170.2 cm	—	22.5 kg/m2	
		65.1		
1/13/2024		63.6	22 kg/m2	Bed Scale

**Estimated needs: **
Weight Used for Equation Calculations: 63.6 kg (140 lb 3.4 oz), Describe Weight Used: Actual

Total Daily Calorie Estimated Needs: 1600-1900 (25-30 kcal/kg) kcal/day
Total Daily Protein Estimated Needs : 65-75 (1-1.2 g/kg) g/day
Total Daily Fluid Estimated Needs: 1600-1900 mL/day

Nutrition Related Summary since last visit:

- Patient with MD at time of visit; interview deferred
- Continues of Regular, Dysphagia 3 diet
- PO intake 69% x 6 charted meals
- Review of meals over past 3 days show average intake of 1712 kcals and 73 grams of protein per day
- Patient
remains on automatic meal tray protocol
- Last BM 1/18; continues to have diarrhea; patient reports diarrhea due to dairy - preferences have been updated
- Hyponatremia noted; nursing monitoring and replacing PRN
- If patient continues to meet estimated needs and updated weight reflects no significant weight loss, reasonable to downgrade to LC2 at next RD visit

**Is patient meeting estimating needs?: **[x] Yes [ ] No
Are current nutrition related orders adequate and appropriate?: [x] Yes [ ] No
Change recommended: N/A

**Plan of Care Discussion: **RN

Nutrition Level of Care: High (LC3)

Monitoring/Evaluation:
PO intake, need for ONS, weight trend, BM/GI sx

Goals:
Patient has a source of nutrition by next RD visit ~ *met*
PO intake to meet GT 75% estimated needs ~ *met/ongoing*

Dakota F., MS, RD
Clinical Nutrition
312-7176

Electronically signed by Your Scripps Health Team at 01/19/2024 1:34 PM PST

Brian M - 01/19/2024 8:26 AM PST
Formatting of this note might be different from the original.

[table#1]:
	01/19/24 0800
Clinical Encounter Type
Visit Date	01/19/24
Visited With	Patient
Routine Visit	Introduction
Referral From	Order
Consult	Patient care
Emotional/Spiritual Services
Assessment	Order was placed for AD. When I arrivedd pts doctor wass working with her. I explained the paper work I was leaving and pt agreed to look over it. i let her know if she has any questions to tell nurse and nurse can call spiritual care dept. for follow up support in filling out paper work. SIlent prayer said for pt as I excused myself. Continued support remains avalible.
Services Provided	Active Listening;Advance Directive Education;Comfort;Ministry of Presence;Prayer;Support/Encouragement
Emotional/Spiritual Outcomes
Outcome	Paper work and education provided for AD for pt. Continued support remains avalible for any questions around AD or spiritual and emtional care and counseling.
Continue Visiting	As needed

Thank you for the opportunity to visit this patient. Please place an "Inpatient consult to Spiritual Care" for future visits as needed. Chaplains remain available.
Brian Henery Meyers, 1/19/2024, 8:26 AM

Electronically signed by Brian M at 01/19/2024 8:26 AM PST

Samuel Ani, DO - 01/19/2024 6:20 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

01/15/2024
Patient seen and examined at bedside. Reported pain in the mouth and some lesions on the tongue.
Patient stated that she usually gets the lesion whenever she is sick. Lesion responds to numbing oral spray,
Per patient. Patient continues to require supplemental oxygen improved from 4 L to 2 L today.

01/16/2024
Continues to have pain in the mouth no reported use of some balm provides some relief. Counseled patient against putting in the mouth because of associated chemicals that could have deleterious effects. Reported she will apply to the lips only. Supplemental oxygen 90 continues to improve currently on 1 L from 2 L 1/15/2024. Mild pain in the lower extremity, right. WBC up trended this morning and patient noted to have an episode of fever spike 1/15/24-concerning for persistent infection in spite of antibiotics. CT chest repeated today showing residual fluid in the right pleural space. Evaluated by pulmonology that plans tPA/DNase intrapleural injection for remaining loculations.

01/17/24
Overnight noted
to have intermittent shortness of breath and generalized body weakness with runs of A-fib/a flutter 110-149 nonsustained asymptomatic. This morning reported improvement of oral soreness. Extremity pain improving with analgesic. Received chest tube flushing as well as tPA/denies intrapleural injection. Much improvement with respect to output 500 mL yesterday and today. Adjustment made to the location of the chest tube. Chest tube site noted to be erythematous. Low urine output (350 mL) in the past 24 hours, with net balance of 1 L. Blood pressure has been soft for which intervention 250 mL of normal saline bolus was made this morning. Patient not a candidate for beta-blockers because of soft blood pressure as well as some recent ejection fraction of less than 30. Will continue to monitor.

01/18/2024
Supplemental oxygen need improved from 2 L NC to 1 L NC. She is net 592.4 mL in the past 24 hours with chest tube drain output of 125 mL. Patient has been afebrile in the past 24 hours. WBC continues to downtrend 11.6 from 12.8. TPA/DNAse on hold by pulmonologist to evaluate the erythema noted at chest tube insertion site. No recorded A-fib/a flutter overnight.

01/19/2024
No acute events overnight. Afebrile. Trial of room air revealed SpO2 of 94% , was put back on 1 L when patient desatted to 93% after about 1 hour. Right foot pain. Continues to need 1 L supplemental oxygen via nasal cannula satting at 100%. She is net +1 L in the last 24 hours, with right chest tube 160 mL also in the last 24 hours, continues to decrease in the last 2 days. WBC normalized to 9.6 from 11.6, first normal value since admission 7 days ago.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 115/87 | Pulse 94 | Temp 36.8 °C (98.2 °F) (Oral) | Resp 19 | Ht 1.702 m (5' 7") | Wt 63.6 kg (140 lb 3.4 oz) | LMP (LMP Unknown) | SpO2 100% | BMI 21.96 kg/m²
I/O: In: 1526 [P.O.:240; I.V.:815.3; IV Piggyback:470.7]
Out: 245 [Urine:125; Chest Tube:120]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. Right chest tube present and functional.
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-**Mild swelling and tenderness of the right foot and deformity of the toes**
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.

Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/19/24
0422	01/18/24
0519	01/15/24
0528	01/14/24
0502	01/14/24
0022	01/13/24
2028	01/13/24
1238	01/13/24
0712	01/13/24
0514	01/12/24
1939
WBC	K/mcL	9.6	**11.6***	< >	**14.3***	--	--	--	--	**21.4***	**18.7***
HGB	g/dL	**10.2***	**10.1***	< >	**9.7***	--	--	--	--	**11.8***	13.2
TOTAL HGB	g/dL	--	--	--	--	--	--	--	**11.1***	--	--
MCV	fL	**102***	**104***	< >	97	--	--	--	--	**101***	99
PLT	K/mcL	**426***	**450***	< >	**573***	--	--	--	--	**801***	**799***
SODIUM	mmol/L	**130***	**132***	< >	**133***	**134***	**132***	< >	--	**133***	137
SODIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	--	**134***	--
--
POTASSIUM	mmol/L	4.7	4.2	< >	4.6	4.6	4.7	< >	--	**6.0***	5.1
POTASSIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	--	**5.3***	--	--
CHLORIDE	mmol/L	100	101	< >	104	104	103	< >	--	101	100
CHLORIDE, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	--	98	--	--
CO2	mmol/L	**32***	30	< >	26	28	27	< >	--	28	**32***
BUN	mg/dL	14	14	< >	**50***	**53***	**56***	< >	--	**55***	**48***
CREATININE	mg/dL	0.60	0.60	< >	**1.20***	**1.22***	**1.30***	< >	--	**1.39***	**1.09***
CALCIUM	mg/dL	**8.2***	**8.3***	< >	**8.1***	**8.3***	**8.0***	< >	--	8.7	9.6
CALCIUM ION	mmol/L	--	--	--	--	--	--	--	1.17	--	--
MAGNESIUM	mg/dL	--	--	--	2.3	--	--	--	--	--	--
PHOSPHORUS	mg/dL	--	--	--	--	2.9	3.5	< >	--	**9.4***	--
GLUCOSE	mg/dL	94	91	< >	**144***	**173***	**135***	< >	--	**235***	**132***
ALBUMIN	g/dL	**2.6***	**2.7***	< >	--	**2.6***	**2.7***	< >	--	**3.4***	3.7
AST	Units/L	24	24	< >	--	--	--	--	--	--	**70***
ALT	Units/L	13	16	< >	--	--	--	--	--	--	31
BILIRUBIN TOTAL	mg/dL	0.3	0.3	< >	--	--	--	--	--	--	0.8
ALK PHOS	Units/L	62	70	< >	--	--	--	--	--	--	**133***
TROPONIN I	ng/mL	--	--	--	--	--	--	--	--	0.014	0.029
CK	Units/L	--	--	--	128	--	--	--	--	**148***	--
NT PRO-BNP	pg/mL	--	--	--	--	--	--	--	--	--	**13,700***
LIPASE	Units/L	--	--	--	--	--	--	--	--	--	114
< > = values in this interval not displayed.

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch

Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Smear	No AFB seen by fluorescent stain
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right;
Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.

X-ray chest single frontal view

Result Date: 1/18/2024
IMPRESSION: Stable pleural-parenchymal opacity the right lung base with no pneumothorax. release CC Dr: p: f:

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath had been started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. Pleural fluid was found to be empyema and patient was intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

#Hypoxic respiratory failure

#Sepsis secondary to right-sided empyema with chest tube placement on 1/13-improving
Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance.
-Extubated 1/14
Strep pneumonia antigen positive 01/14
Chest CT performed 1/16/2024 showed residual right pleural fluid.
-**Wean supplemental oxygen as tolerated.**
-Normal white count 9.6 on 1/19/2024
-Right chest tube output 160 mL in the last 24 hours, output continues to decrease in the last 48 hours
-Patient followed by pulmonology, Dr. Dammert, appreciate recs
-Pleural fluid culture collected 1/13, no growth to date
-AFB culture with stain performed on 01/13/2024-no AFB stain
-Blood culture, collected 01/12/2024, no growth to date
-Influenza A/B and COVID-19 rapid PCR/NAAT-both negative
-Continue Rocephin and Flagyl (1/12-)

#A-fib RVR (CHA2DS2-VASc score of 2)
-S/p amnio gtt.
Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic. 1/17/2024, night, was negative for A-fib/a flutter
-Consider amiodarone if if A-fib/flutter continues
-Obtain twelve-lead EKG
-Will continue gentle rehydration.
-Recommend outpatient follow up to discuss anticoagulant.

#HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation
-Holding Coreg and lisinopril given soft BP
-When appropriate initiate low-dose carvedilol.

#AKI-resolved
Creatinine 1.2 OA, from baseline of 0.9.
Normal value 1/15/2024
Continue to avoid nephrotoxic medications

#Hyponatremia
Restriction to 1.5 L daily
Continue to monitor serum sodium

#Right ankle/foot pain
Mild swelling and tenderness of the right foot and deformity of the toes ongoing for 3 weeks
Likely due to arthritis, rule out gout
-Right foot x-ray
-follow up uric acid
-possible colchicine

#Transaminitis
AST of 102
Continue to monitor

 # Bronchial Asthma
 # Possible COPD.
-DuoNebs as needed

 # Mouth pain and ulcers-much improvement
-Likely oral herpes
-Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed le
sions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray.
-Continue IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.
-Continue oral clotrimazole

Updated daughter, Erika, 1/19/2024. Erika stated that she makes decisions for the family regarding the patient's health. She wanted the social worker to help the patient get into San diego shelter.
Samuel Ani, DO
Internal medicine resident ** **

Attending: Dr. Dao, Diep Ngoc, DO

Cosigned by Diep Dao, DO at 01/19/2024 9:26 PM PST

Electronically signed by Samuel Ani, DO at 01/19/2024 11:45 AM PST
Electronically signed by Samuel Ani, DO at 01/19/2024 12:28 PM PST
Electronically signed by Samuel Ani, DO at 01/19/2024 12:36 PM PST
Electronically signed by Samuel Ani, DO at 01/19/2024 7:05 PM PST
Electronically signed by Diep Dao, DO at 01/19/2024 9:26 PM PST

Associated attestation - Diep Dao, DO - 01/19/2024 9:26 PM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note above as needed. I agree with and verify the findings and plan

Samuel Ani, DO - 01/18/2024 2:59 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

01/15/2024
Patient seen and examined at bedside. Reported pain in the mouth and some lesions on the tongue.
Patient stated that she usually gets the lesion whenever she is sick. Lesion responds to numbing oral spray,
Per patient. Patient continues to require supplemental oxygen improved from 4 L to 2 L today.

01/16/2024
Continues to have pain in the mouth no reported use of some balm provides some relief. Counseled patient against putting in the mouth because of associated chemicals that could have deleterious effects. Reported she will apply to the lips only. Supplemental oxygen 90 continues to improve currently on 1 L from 2 L 1/15/2024. Mild pain in the lower extremity, right. WBC up trended this morning and patient noted to have an episode of fever spike 1/15/24-concerning for persistent infection in spite of antibiotics. CT chest repeated today showing residual fluid in the right pleural space. Evaluated by pulmonology that plans tPA/DNase intrapleural injection for remaining loculations.

01/17/24
Overnight noted to have intermittent shortness of breath and generalized body weakness with runs of A-fib/a flutter 110-149 nonsustained asymptomatic. This morning reported improvement of oral soreness. Extremity pain improving with analgesic. Received chest tube flushing as well as tPA/denies intrapleural injection. Much improvement with respect to output 500 mL yesterday and today. Adjustment made to the location of the chest tube. Chest tube site noted to be erythematous. Low urine output (350 mL) in the past 24 hours, with net balance of 1 L. Blood pressure has been soft for which intervention 250 mL of normal saline bolus was made this morning. Patient not a candidate for beta-blockers because of soft blood pressure as well as some recent ejection fraction of less than 30. Will continue to monitor.

01/18/2024
Supplemental oxygen need improved from 2 L NC to 1 L NC. She is net 592.4 mL in the past 24 hours with chest tube drain output of 125 mL. Patient has been afebrile in the past 24 hours. WBC continues to downtrend 11.6 from 12.8. TPA/DNAse on hold by pulmonologist to evaluate the erythema noted at chest tube insertion site. No recorded A-fib/a flutter overnight.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 105/73 (BP Location: Left arm, Patient Position: Lying) | Pulse 108 | Temp 37.5 °C (99.5 °F) (Axillary) | Resp 22 | Ht 1.702 m (5' 7") | Wt 63.6 kg (140 lb 3.4 oz) | LMP (LMP Unknown)

| SpO2 93% | BMI 21.96 kg/m²
I/O: In: 997.4 [P.O.:480; I.V.:401.7; IV Piggyback:115.7]
Out: 405 [Urine:125; Chest Tube:280]
General-well-developed, well-nourished, awake and alert
HEENT-normocephalic atraumatic
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. Right chest tube present and functional.
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-normal bulk and tone
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/18/24
0519	01/17/24
0519	01/15/24
0528	01/14/24
0502	01/14/24
0022	01/13/24
2028	01/13/24
1238	01/13/24
0712	01/13/24
0514	01/12/24
1939
WBC	K/mcL	**11.6***	**12.8***	< >	**14.3***	--	--	--	--	**21.4***	**18.7***
HGB	g/dL	**10.1***	**10.4***	< >	**9.7***	--	--	--	--	**11.8***	13.2
TOTAL HGB	g/dL	--	--	--	--	--	--	--	**11.1***	--	--
MCV	fL	**104***	**102***	< >	97	--	--	--	--	**101***	99
PLT	K/mcL	**450***	**509***	< >	**573***	--	--	--	--	**801***	**799***
SODIUM	mmol/L	**132***	**132***	< >	**133***	**134***	**132***	< >	--	**133***	137
SODIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	--	**134***	--	--
POTASSIUM	mmol/L	4.2	4.3	< >	4.6	4.6	4.7	< >	--	**6.0***	5.1
POTASSIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	--	**5.3***	--	--
CHLORIDE	mmol/L	101	99	< >	104	104	103	< >	--	101	100
CHLORIDE, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	--	98	--	--
CO2	mmol/L	30	**32***	< >	26	28	27	< >	--	28	**32***
BUN	mg/dL	14	**19***	< >	**50***	**53***	**56***	< >	--	**55***	**48***
CREATININE	mg/dL	0.60	0.70	< >	**1.20***	**1.22***	**1.30***	< >	--	**1.39***	**1.09***
CALCIUM	mg/dL	**8.3***	8.6	< >	**8.1***	**8.3***	**8.0***	< >	--	8.7	9.6
CALCIUM ION	mmol/L	--	--	--	--	--	--	--	1.17	--	--
MAGNESIUM	mg/dL	--	--	--	2.3	--	--	--	--	--	--
PHOSPHORUS	mg/dL	--	--	--	--	2.9	3.5	< >	--	**9.4***	--
GLUCOSE	mg/dL	91	99	< >	**144***	**173***	**135***	< >	--	**235***	**132***
ALBUMIN	g/dL	**2.7***	**2.5***	< >	--	**2.6***	**2.7***	< >	--	**3.4***	3.7
AST	Units/L	24	24	< >	--	--	--	--	--	--	**70***
ALT	Units/L	16	16	< >	--	--	--	--	--	--	31
BILIRUBIN TOTAL	mg/dL	0.3	0.3	< >	--	--	--	--	--	--	0.8
ALK PHOS	Units/L	70	77	< >	--	--	--	--	--	--	**133***
TROPONIN I	ng/mL	--	--	--	--	--	--	--	--	0.014	0.029
CK	Units/L	--	--	--
128	--	--	--	--	**148***	--
NT PRO-BNP	pg/mL	--	--	--	--	--	--	--	--	--	**13,700***
LIPASE	Units/L	--	--	--	--	--	--	--	--	--	114
< > = values in this interval not displayed.

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM

	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Smear	No AFB seen by fluorescent stain
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.

X-ray chest single frontal view

Result Date: 1/18/2024
IMPRESSION: Stable pleural-parenchymal opacity the right lung base with no pneumothorax. release CC Dr: p: f:

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath had been started 2 days prior to presentation with cough, fever, dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. Pleural fluid was found to be empyema and patient was intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. While in the ICU she was noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

#Hypoxic respiratory failure
#Sepsis secondary to right-sided empyema with chest tube placement on 1/13
Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance.
-Extubated 1/14
Strep pneumonia antigen positive 01/14
Chest CT performed 1/16/2024 showed residual right pleural fluid. Patient evaluated by
-PI
eural fluid culture collected 1/13, no growth to date
-AFB culture with stain performed on 01/13/2024-no AFB stain
-Blood culture, collected 01/12/2024, no growth to date
-Influenza A/B and COVID-19 rapid PCR/NAAT-both negative
-Continue Rocephin and Flagyl (1/12-)

#A-fib RVR (CHA2DS2-VASc score of 2)
-S/p amnio gtt.
Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic. 1/17/2024, night, was negative for A-fib/a flutter
-Consider amiodarone if if A-fib/flutter continues
-Obtain twelve-lead EKG
-Will continue gentle rehydration.
-Recommend outpatient anticoagulant.

#HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation
-Holding Coreg and lisinopril given soft BP
-When appropriate initiate low-dose carvedilol.

#AKI-resolved
Creatinine 1.2 from baseline of 0.9.
Normal value 1/15/2024
Continue to avoid nephrotoxic medications

#Hyponatremia
Continue to monitor serum sodium

#Transaminitis
AST of 102
Continue to monitor

#Asthma
-DuoNebs as needed

 # Mouth pain and ulcers-much improvement
-Likely oral herpes
-Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely that patient also has oral candidiasis
Patient also reported benefit with lidocaine spray.
-Continue IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.
-Continue oral clotrimazole

Samuel Ani, DO
Internal medicine resident ** **

Attending: Dr. Dao, Diep Ngoc, DO

Cosigned by Diep Dao, DO at 01/18/2024 9:45 PM PST
Electronically signed by Samuel Ani, DO at 01/18/2024 3:15 PM PST
Electronically signed by Diep Dao, DO at 01/18/2024 9:45 PM PST

Associated attestation - Diep Dao, DO - 01/18/2024 9:45 PM PST
Formatting of this note might be different from the original.
I have personally evaluated and examined the patient, reviewed the labs and pertinent studies. The patient was seen and discussed with the Resident. The Resident assessment and documentation on this patient was reviewed in detail. The physical exam and the assessment and plan have been independently performed by me and I have edited the note above as needed. I agree with and verify the findings and plan

Pedro Dammert Coronado, MD - 01/18/2024 2:55 PM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista

Chief complain:
Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

History of Present Illness:
This is hospital day 6 for Elizabeth Tina Petruccio a 59 y/o woman admitted with an empyema

Subjective:
Less SOB
Less pain around chest tube insertion site

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
Scheduled Meds:acetaminophen, 650 mg, oral, Q8H
acyclovir, 400 mg, oral, Q8H SCH
cefTRIAXone, 2 g, intraVENOUS, Q24H
clotrimazole, 10 mg, mucous membrane, TID
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
enoxaparin, 40 mg, subcutaneous, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID
sodium chloride, 250 mL, intraVENOUS, Once

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• ipratropium-albuteroL, nebulization, Q4H PRN, Ho, Truc Viet, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian
, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Allergies on file: Patient has no known allergies.

Review of Systems:
10 organ review of systems was negative other than what mentioned in HPI

**Family History: **
[table#2]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Social History:
[table#3]:
Social History

[table#4]:
Socioeconomic History
•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	Not on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance and Sexual Activity
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#5]:
Social Determinants of Health

[table#6]:
Financial Resource Strain: Not on file
Food Insecurity: Unknown (1/3/2024)
	Food Insecurity
	•	In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Not on file
Transportation Needs: Unknown (1/3/2024)
	Transportation Needs
	•	In the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Not on file
Physical Activity: Not on file
Stress: Not on file
Social Connections: Not on file
Intimate Partner Violence: Not on file
Housing Stability: Not on file

Physical Exam:
[table#7]:
Vitals:
	01/18/24 1357
BP:	
Pulse:	
Resp:	
Temp:	
SpO2:	93%

[table#8]:

| Temp: [36.9 °C (98.4 °F)-37.5 °C (99.5 °F)] 37.5 °C (99.5 °F)
Heart Rate: [93-108] 108
Resp: [12-27] 22
BP: (92-128)/(63-78) 105/73
FiO2 (%): [21 %-24 %] 21 % | SpO2: 93 %
FiO2 (%): 21 %
O2 Flow Rate (L/min): 2 L/min | Height: 170.2 cm (5' 7")
Weight: 63.6 kg (140 lb 3.4 oz)
BMI (Calculated): 22 |
| --- | --- | --- |
Cardiac Rhythm: Sinus tachycardia
In: 997.4 [P.O.:480; I.V.:401.7; IV Piggyback:115.7]
Out: 405 [Urine:125; Chest Tube:280]

Intake/Output Summary (Last 24 hours) at 1/18/2024 1455
Last data filed at 1/18/2024 1200
[table#9]:
Gross per 24 hour
Intake	1678.23 ml
Output	405 ml
Net	**1273.23 ml**

[table#10]:
Last 5 weights
	01/12/24 1931	01/13/24 0205
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)

FiO2 (%): [21 %-24 %] 21 %

EXAM:
GEN: awake, in mld distress
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist mucous membranes, oral ulcers
HEART: RRR, no murmur
LUNGS: diminished in the R base, chest tube in place without erythema
ABD: soft, NT, ND,
EXT/MSK: no LE edema,
NEURO: awake and alert, moving all 4 ext to command

**Laboratory Data: ** Personally Reviewed

**Imaging: ** Personally Reviewed

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure
2. Right sided empyema
3. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes

Recommendations:
- Chest tube insertion site not erythematous this morning. Chest tube flushed
- CXR shows that lung re-expanded
- Keep chest tube to suction
- No need for
surgical decortication as the lung re-expanded well
- Continue cefriaxone/flagyl
- Pain control

Pedro Manuel Dammert Coronado, MD, 1/18/2024, 2:55 PM
Pulmonary and Critical Care Medicine
#8086509

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest

imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system. Patient's risk is high.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Pedro Dammert Coronado, MD at 01/18/2024 2:56 PM PST

Sandra S - 01/18/2024 9:28 AM PST

Summary: oxygen rounds

Formatting of this note might be different from the original.

[table#1]:
	01/18/24 0908
Oxygen Therapy/Pulse Ox
Oxygen Therapy	(S) None (Room air)
O2 Delivery Method	(S) Other (Comment)
O2 Flow Rate (L/min)	(S) 0 L/min
FiO2 (%)	(S) 21 %
Patient Activity	(S) At rest
$Oxygen Initiated/Discontinued	(S) Discontinued
$ Pulse Oximetry Spot Check	(S) Yes
Skin Assessment
Skin Assessment	(S) WDL

Pt weaned to room air. W Spo2 of 93%
While at bedside. Pt insisted on using lidocaine solution she had at bedside for oral application. C/o of burning sensation in mouth and lips.

Oxygen documention was updated. Oxygen charge was d/c

Sandra M. Sandoval-Medina, RT, 1/18/2024, 9:32 AM

Electronically signed by Sandra S at 01/18/2024 9:32 AM PST

Pedro Dammert Coronado, MD - 01/17/2024 11:45 AM PST

Formatting of this note might be different from the original.

Pulmonary Medicine Attending Progress Note - Scripps Chula Vista

Chief complain:
Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

History of Present Illness:
This is hospital day 5 for Elizabeth Tina Petruccio a 59 y/o woman admitted with an empyema

Subjective:
C/o mild SOB
C/o pain around chest tube insertion site

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
Scheduled Meds:acetaminophen, 650 mg, oral, Q8H
acyclovir, 400 mg, oral, Q8H SCH
cefTRIAXone, 2 g, intraVENOUS, Q24H
clotrimazole, 10 mg, mucous membrane, TID
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
enoxaparin, 40 mg, subcutaneous, Daily

lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID
sodium chloride, 250 mL, intraVENOUS, Once
sodium chloride, 250 mL, intraVENOUS, Once

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• ipratropi
um-albuteroL, nebulization, Q4H PRN, Ho, Truc Viet, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• naproxen, oral, BID PRN, Ani, Samuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Allergies on file: Patient has no known allergies.

Review of Systems:
10 organ review of systems was negative other than what mentioned in HPI

**Family History: **
[table#2]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Social History:
[table#3]:
Social History

[table#4]:
Socioeconomic History
•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	Not on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance and Sexual Activity
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#5]:
| **Social Determinants of Health** |

| --- |

[table#6]:
Financial Resource Strain: Not on file
Food Insecurity: Unknown (1/3/2024)
	Food Insecurity
	•	In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Not on file
Transportation Needs: Unknown (1/3/2024)
	Transportation Needs
	•	In the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Not on file
Physical Activity: Not on file
Stress: Not on file
Social Connections: Not on file
Intimate Partner Violence: Not on file
Housing Stability: Not on file

Physical Exam:
[table#7]:
Vitals:
	01/17/24 1104
BP:	91/68
Pulse:	100
Resp:	20
Temp:	37.3 °C (99.1 °F)
SpO2:	96%

[table#8]:
| Temp: [36.9 °C (98.4 °F)-37.3 °C (99.1 °F)] 37.3 °C (99.1 °F)
Heart Rate: [78-148] 100
Resp: [17-24] 20
BP: (81-119)/(56-76) 91/68
FiO2 (%): [24 %-28 %] 24 % | SpO2: 96 %
FiO2 (%): 24 %
O2 Flow Rate (L/min): 1 L/min | Height: 170.2 cm (5' 7")
Weight: 63.6 kg (140 lb 3.4 oz)
BMI (Calculated): 22 |
| --- | --- | --- |
Cardiac Rhythm: Atrial fibrillation, Atrial flutter
In: 2430.3 [P.O.:860; I.V.:27.8; IV Piggyback:1542.5]
Out: 1360 [Urine:350; Chest Tube:1010]

Intake/Output Summary (Last 24 hours) at 1/17/2024 1145
Last data filed at 1/17/2024 0625
[table#9]:
Gross per 24 hour
Intake	2070.28 ml
Output	1350 ml
Net	**720.28 ml**

[table#10]:
Last 5 weights
	01/12/24 1931	01/13/24 0205
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)

FiO2 (%): [24 %-28 %] 24 %

EXAM:
GEN: awake, in mld distress
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist mucous membranes, oral ulcers
HEART: RRR, no murmur
LUNGS: diminished in the R base, chest tube in place
ABD: soft, NT, ND,

EXT/MSK: no LE edema,
NEURO: awake and alert, moving all 4 ext to command

**Laboratory Data: ** Personally Reviewed

**Imaging: ** Personally Reviewed

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure
2. Right sided empyema
3. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes

Recommendations:
- Chest tub
e insertion site erythemarous this morning. Unclear if it is from TPA/DNAse or cellulitis. Will hold on any more TPA/DNAse for now.
CXR shows that lung re-expanded
- Keep chest tube to suction
- No need for surgical decortication as the lung re-expanded well
- Continue cefriaxone/flagyl
- Pain control

Pedro Manuel Dammert Coronado, MD, 1/17/2024, 11:45 AM
Pulmonary and Critical Care Medicine
#8086509

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system. Patient's risk is high.

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Electronically signed by Pedro Dammert Coronado, MD at 01/17/2024 11:47 AM PST

Samuel Ani, DO - 01/17/2024 11:19 AM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

01/15/2024
Patient seen and examined at bedside. Reported pain in the mouth and some lesions on the tongue.
Patient stated that she usually gets the lesion whenever she is sick. Lesion responds to numbing oral spray,
Per patient. Patient continues to require supplemental oxygen improved from 4 L to 2 L today.

01/16/2024
Continues to have pain in the mouth no reported use of some balm provides some relief. Counseled patient against putting in the mouth because of associated chemicals that could have deleterious effects. Reported she will apply to the lips only. Supplemental oxygen 90 continues to improve currently on 1 L from 2 L 1/15/2024. Mild pain in the lower extremity, right. WBC up trended this morning and patient noted to have an episode of fever spike 1/15/24-concerning for persistent infection in spite of antibiotics. CT chest repeated today showing residual fluid in the right pleural space. Evaluated by pulmonology that plans tPA/DNase intrapleural injection for remaining loculations.

01/17/24
Overnight noted to have Noted to have intermittent shortness of breath and generalized body weakness with runs of A-fib/a flutter 110-149 nonsustained asymptomatic. This morning reported improvement of oral soreness. Extremity pain improving with analgesic. Received chest tube flushing as well as tPA/denies intrapleural injection. Much improvement with respect to output 500 mL yesterday and today. Adjustment made to the location of the chest tube. Chest tube site noted to be erythematous. Low urine output (350 mL) in the past 24 hours, with net balance of 1 L. Blood pressure has been soft for which intervention 250 mL of normal saline bolus was made this morning. Patient not a candidate for beta-blockers because of soft blood pressure as well as some recent ejection fraction of less than 30. Will continue to monitor.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 91/68 (BP Location: Left arm, Patient Position: Lying) | Pulse 100 | Temp 37.3 °C (99.1 °F) (Oral) | Resp 20 | Ht 1.702 m (5' 7") | Wt 63.6 kg (140 lb 3.4 oz) | LMP (LMP Unknown) | SpO2 96% | BMI 21.96 kg/m²
I/O: In: 2430.3 [P.O.:860; I.V.:27.8; IV Piggyback:1542.5]
Out: 1360 [Urine:350; Chest Tube:1010]
General-well-developed, well-nourished, drowsy-perhaps due to sedative that she received last night. In
HEENT-normocephalic atraumatic
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. Right chest tube present and functional.
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-normal bulk and tone
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/17/24
0519	01/16/24
0519	01/15/24
0528	01/14/24
0502	01/14/24
0022	01/13/24
2028	01/13/24
1238	01/13/24
0712	01/13/24
0514	01/12/24
1939
WBC	K/mcL	**12.8***	**13.1***	< >	**14.3***	--	--	--	--	**21.4***	**18.7***
HGB	g/dL	**10.4***	**10.6***	< >	**9.7***	--	--	--	--	**11.8***	13.2
TOTAL HGB	g/dL	--	--	--	--	--	--	--	**11.1***	--	--
MCV	fL	**102***	**104***	< >	97	--	--	--	--	**101***	99
PLT	K/mcL	**509***	**546***	< >	**573***	--	--	--	--	**801***	**799***
SODIUM	mmol/L	**132***	**131***	< >	**133***	**134***	**132***	< >	--	**133***	137
SODIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	--	**134***	--	--
POTASSIUM	mmol/L	4.3	4.6	< >	4.6	4.6	4.7	< >	--	**6.0***	5.1
POTASSIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	--	**5.3***	--	--
CHLORIDE	mmol/L	99	100	< >	104	104	103	< >	--	101	100
CHLORIDE, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	--	98	--	--
CO2	mmol/L	**32***	**37***	< >	26	28	27	< >	--	28	**32***
BUN	mg/dL	**19***	**20***	< >	**50***	**53***	**56***	< >	--	**55***	**48***
CREATININE	mg/dL	0.70	0.70	< >	**1.20***	**1.22***	**1.30***	< >	--	**1.39***	**1.09***

CALCIUM	mg/dL	8.6	8.5	< >	**8.1***	**8.3***	**8.0***	< >	--	8.7	9.6
CALCIUM ION	mmol/L	--	--	--	--	--	--	--	1.17	--	--
MAGNESIUM	mg/dL	--	--	--	2.3	--	--	--	--	--	--
PHOSPHORUS	mg/dL	--	--	--	--	2.9	3.5	< >	--	**9.4***	--
GLUCOSE	mg/dL	99	116	< >	**144***	**173***	**135***	< >	--	**235***	**132***
ALBUMIN	g/dL	**2.5***	**2.8***	--	--	**2.6***	**2.7***	< >	--	**3.4***	3.7
AST	Units/L	24	**102***	--	--	--	--	--	--	--	**70***
ALT	Units/L	16	19	--	--	--
--	--	--	--	31
BILIRUBIN TOTAL	mg/dL	0.3	0.4	--	--	--	--	--	--	--	0.8
ALK PHOS	Units/L	77	95	--	--	--	--	--	--	--	**133***
TROPONIN I	ng/mL	--	--	--	--	--	--	--	--	0.014	0.029
CK	Units/L	--	--	--	128	--	--	--	--	**148***	--
NT PRO-BNP	pg/mL	--	--	--	--	--	--	--	--	--	**13,700***
LIPASE	Units/L	--	--	--	--	--	--	--	--	--	114
< > = values in this interval not displayed.

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth to Date
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Smear	No AFB seen by fluorescent stain
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final

| | Gram Stain | Moderate WBCs |
| | Gram Stain | No organisms seen |
| **Strep pneumo antigen, urine** |
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)

X-ray portable chest single frontal view

Result Date: 1/17/2024
IMPRESSION: Stable exam, overall no significant change release

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. release was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

#Hypoxic respiratory failure
#Sepsis secondary to right-sided empyema with chest tube placement on 1/13
Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance.
-Extubated 1/14
Strep pneumonia
antigen positive
Chest CT performed 1/16/2024 showed residual right pleural fluid. Patient evaluated by pulmonology today, Dr. Dammert, who has ordered tPA/DNase
-Thoracentesis with exudative fluid
-pleural fluid culture collected 1/13, no growth to date
-AFB culture with stain performed on 01/13/2024-no AFB stain
-Blood culture, collected 01/12/2024, no growth to date
-Influenza A/B and COVID-19 rapid PCR/NAAT-both negative
-Continue chest tube to waterseal
-Right chest tube in place draining
-Currently on Rocephin, Flagyl ,(1/12-)

#A-fib RVR
-S/p amnio gtt.
Telemetry monitoring-positive for runs of A-fib/a flutter last night, 11/16/2024 but asymptomatic.
-Consider amiodarone if he continues
-Obtain twelve-lead EKG
-Will continue gentle rehydration.

#HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation
-Holding Coreg and lisinopril given soft BP
-When appropriate initiate low-dose carvedilol.

#AKI-resolved
Creatinine 1.2 from baseline of 0.9.
Normal value 1/15/2024
Continue to avoid nephrotoxic medications

#Hyponatremia
Continue to monitor serum sodium

#Transaminitis
AST of 102
Continue to monitor

#Asthma
-DuoNebs as needed

 # Mouth pain and ulcers-much improvement

-Likely oral herpes
-Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely patient is having oral herpes.
Patient also reported benefit with lidocaine spray.
-start IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.

Samuel Ani, DO
Internal medicine resident ** **

Attending: Dr. Strauser, Martha Dalia, MD

Cosigned by Martha Strauser, MD at 01/17/2024 1:45 PM PST
Electronically signed by Samuel Ani, DO at 01/17/2024 11:31 AM PST
Electronically signed by Martha Strauser, MD at 01/17/2024 1:45 PM PST

Your Scripps Health Team - 01/17/2024 9:44 AM PST
Formatting of this note might be different from the original.
**Clinical Nutrition Monitor Note **

Recommendations to Provider:
1. Continue Regular diet, Dysphagia 3 per SLP
2. Continue daily MVT with minerals

**Nutrition Diagnosis (PES statement): **
Inadequate oral Intake* *related to previous mechanical ventilation, now extubated with PO diet as evidenced by PO intake 75%, meeting less than 75% estimated kcal needs ~ *revised/ongoing*

[table#1]:
|
|
| --- |
| [table#1.1] |

[table#1.1]:
Dietary Orders
(From admission, onward)

[table#2]:
|
|
| --- |

[table#3]:
| | [table#3.1]
[table#3.2] |
| --- | --- |

[table#3.1]:
	Start			Ordered
	01/15/24 1048		[table#3.1.1]	01/15/24 1047

[table#3.1.1]:
Combination Diet Regular; Dysphagia 3 Advanced Texture Diet effective now
[table#3.1.1.1]

[table#3.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Food Consistency:	Dysphagia 3 Advanced Texture	

[table#3.2]:

[table#4]:

Food Allergies: NKFA

Nutritionally significant labs:
[table#5]:
Results from last 7 days
Lab	Units	01/17/24
0519	01/16/24
0519	01/15/24
0528
SODIUM	mmol/L	**132***	**131***	**135***

Nutritionally significant medications:
- enoxaparin, 40 mg, subcutaneous, Daily
- multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
- sennosides-docusate sodium, 1 tablet, oral, BID

Skin: Intact

**Anthropometrics: **
Height: 170.2 cm (5' 7"), Weight: 63.6 kg (140 lb 3.4 oz), BMI (Calculated): 22

Weight trend this admission: [ ] Loss[ ] Gain [ ] Stable [x] Requested new weight
Weight change assessment:
[table#6]:
	Date			
	Height	Weight	BMI \
(Calculated)	Weight Method
1/12/2024	170.2 cm	—	22.5 kg/m2	
		65.1		
1/13/2024		63.6	22 kg/m2	Bed Scale

**Estimated needs: **
Weight Used for Equation Calculations: 63.6 kg (140 lb 3.4 oz), Describe Weight Used: Actual
Total Daily Calorie Estimated Needs: 1600-1900 (25-30 kcal/kg) kcal/day
Total Daily Protein Estimated Needs : 65-75 (1-1.2 g/kg) g/day
Total Daily Fluid Estimated Needs: 1600-1900 mL/day

Nutrition Related Summary since last visit:
- Patient seen at bedside; denies any n/v but per patient and RN, has been having loose BMs
- Diet hx obtained at this visit
- Continues on Regular, dysphagia 3 diet per SLP
- PO intake 75% x 3 charted meals
- Review of meals over past 2 days show average intake of 1190 kcals and 59 grams of protein per day meeting 74/91% estimated needs
- Declines addition of oral nutrition supplement (ONS)
- New food preferences obtained and updated in CBORD
- Patient remains on automatic meal protocol
- Patient reports mouth sores are impairing her ability to eat; MD and RN aware
- Chest tube with high output yesterday 1/16 of 1010 mL
- Hyponatremia noted; nursing monitoring and replacing PRN

**Is patient meeting estimating needs?: **[ ] Yes [x] No
Are current nutrition related orders adequate and appropriate?: [ ] Yes [ ] No
Change recommended: N/A

**Plan of Care Discussion: **Patient and RN

Nutrition Level of Care: High (LC3)

Monitoring/Evaluation:
PO intake, need for ONS, weight trend, BM, BG

Goals:
Patient has a source of nutrition by next RD visit ~ *met*
PO intake to meet GT 75% estimated needs ~ *ongoing*

Dakota F., MS, RD
Clinical Nutrition
312-7176

Electronically signed by Your Scripps Health Team at 01/17/2024 12:17 PM PST

Martha Strauser, MD - 01/17/2024 8:33 AM PST
Formatting of this note might be different from the original.
****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
59 female with history of alcohol abuse and remote methamphetamine abuse with 2 drug screen test negative on 2021 and on admission. Has history of cardiomyopathy with reduced ejection fraction of 26%. Admitted with shortness of breath and cough. Positive urine strep antigen. On chest x-ray found to have a right pleural effusion with atelectasis. CT corroborated pleural effusion. Status postthoracentesis with fluid positive for bacteria and compatible with exudate. Has been on Rocephin Flagyl and vancomycin. Of concern as well complaining of herpesvirus flare with severe ulcerations in her mouth. She explains to me whenever she is under stress herpesvirus flares and has been under a lot of stress complaining of horrible mouth pain where we can identify multiple diabetic ulcers.
On admission creatinine 1.2 back to 0.9 after hydration.
Chest tube has draining over 1000 cc in last 24 hours of clearly purulent discharge.Chest tube was flushed January 16 and was a started in intrapleural tPA/ DNase
.**Objective**
[table#2]:
**OBJECTIVE **
Temp: 37 °C (98.6 °F)
Heart Rate: 94
Resp: 22
BP: 87/63
SpO2: 97 %
FiO2 (%): 26 %
O2 Flow Rate (L/min): 1.5 L/min
Temp (24hrs), Avg:37.1 °C (98.7 °F), Min:36.9 °C (98.4 °F), Max:37.4 °C (99.3 °F)

SpO2: 97 %
FiO2 (%): 26 %
O2 Flow Rate (L/min): 1.5 L/min
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND
Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs
[table#3]:
Results from last 7 days
Lab	Units	01/17/24
0519	01/16/24
0519	01/15/24
0528	01/13/24
0514	01/12/24
1939
SODIUM	mmol/L	**132***	**131***	**135***	< >	137
SODIUM, WHOLE BLOOD		--	--	--	< >	--

POTASSIUM	mmol/L	4.3	4.6	4.2	< >	5.1
POTASSIUM, WHOLE BLOOD		--	--	--	< >	--
CHLORIDE	mmol/L	99	100	102	< >	100
CHLORIDE, WHOLE BLOOD	
--	--	--	< >	--
CO2	mmol/L	**32***	**37***	**34***	< >	**32***
BUN	mg/dL	**19***	**20***	**33***	< >	**48***
CREATININE	mg/dL	0.70	0.70	0.90	< >	**1.09***
CALCIUM	mg/dL	8.6	8.5	8.4	< >	9.6
BILIRUBIN TOTAL	mg/dL	0.3	0.4	--	--	0.8
ALK PHOS	Units/L	77	95	--	--	**133***
ALT	Units/L	16	19	--	--	31
AST	Units/L	24	**102***	--	--	**70***
GLUCOSE	mg/dL	99	116	102	< >	**132***
< > = values in this interval not displayed.

[table#4]:
Results from last 7 days
Lab	Units	01/17/24
0519	01/16/24
0519	01/15/24
0528
WBC	K/mcL	**12.8***	**13.1***	**12.5***
HGB	g/dL	**10.4***	**10.6***	**10.4***
HEMATOCRIT	%	**34.4***	**35.1***	**33.9***
PLT	K/mcL	**509***	**546***	**543***
NEUTROS PCT AUTO	%	**86.1***	**85.4***	**85.7***
LYMPHS PCT AUTO	%	**7.6***	**8.1***	**7.5***
MONOS PCT AUTO	%	4.9	**4.1***	**3.8***
EOS PCT AUTO	%	0.2	0.2	0.2

[table#5]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/13/24
0514	01/12/24
1939
CK	Units/L	128	**148***	--
TROPONIN I	ng/mL	--	0.014	0.029

[table#6]:
Results from last 7 days
Lab	Units	01/12/24
1939
NT PRO-BNP	pg/mL	**13,700***

[table#7]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final
01/14/2024 05:17 PM	93	70 - 125 mg/dL	Final
01/14/2024 12:29 PM	101	70 - 125 mg/dL	Final

[table#8]:
Recent Results (from the past 168 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	Influenza A PCR	Not Detected	Not Detected
	Influenza B PCR	Not Detected	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM

	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value	Ref Range
	Urine Culture		
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value	Ref Range
	Blood Culture	No Growth to Date	
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value	Ref Range
	Blood Culture	No Growth to Date	
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value	Ref Range
	Staph Screen culture		
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value	Ref Range
	AFB Smear	No AFB seen by fluorescent stain	
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value	Ref Range
	Wound Culture	No Growth to Date	
	Gram Stain	Many WBCs	
	Gram Stain	No organisms seen	
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value	Ref Range
	Culture	No Growth Final	
Gram Stain	Moderate WBCs	
	Gram Stain	No organisms seen	
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value	Ref Range
	Strep Pneumo Ag,UR	Positive (A)	Negative

Imaging Studies
Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 1/16/2024
IMPRESSION: Negative for deep vein thrombosis in both lower extremities. release

CT chest without contrast for pneumonia STAT

Result Date: 1/16/2024
IMPRESSION: 1. Small to moderate right pleural effusion with indwelling pleural drainage catheter. Subjacent right basilar atelectasis/consolidation 2. Right upper lobe groundglass opacity with peripheral consolidation (reverse halo sign). Findings can be seen in the setting of atypical community-acquired bacterial pneumonia, fungal pneumonia, and organizing pneumonia. release

Current Scheduled Medications:
acetaminophen, 650 mg, oral, Q8H
acyclovir, 400 mg, oral, Q8H SCH
cefTRIAXone, 2 g, intraVENOUS, Q24H
clotrimazole, 10 mg, mucous membrane, TID
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime

enoxaparin, 40 mg, subcutaneous, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID
sodium chloride, 250 mL, intraVENOUS, Once
sodium chloride, 250 mL, intraVENOUS, Once

Current Infusion Medications:
dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 01/17/24 0658
ipratropium-albuteroL, 3 mL, Q4H PRN, 3 mL at 01/17/24 0536
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/16/24 0251
melatonin, 10 mg, At bedtime PRN, 10 mg at 01/16/24 2145
ondansetron, 4 mg, Q6H PRN, 4 mg at 01/13/24 0030
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
naproxen, 500 mg, BID PRN
zolpidem, 5 mg, At bedtime PRN, 5 mg at 01/15/24 2215

Assessment/Plan
[table#9]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with remote history of methamphetamine abuse and alcohol abuse with cardiomyopathy with reduced ejection fraction. Admitted with shortness of breath and pneumonia with right pleural effusion. Status post intubation. Extubated on January 14. Status post thoracentesis with evidence of exudative growing bacteria. Urine strep antigen positive. On Rocephin vancomycin and metronidazole awaiting final culture and sensitivity results. From the point of view of pneumonia feels better. Complaining of severe oral pain due to multiple oral ulcers. Has been diagnosed with herpes and this is one of her bad flares
-Right lung pneumonia with pleural exudates suggestive of parapneumonic effusion /empyema. Chest tube appears to be draining yellowish purulent fluid. Now on tPA/DNase if fluid draining stops may require further interventions including cardiothoracic surgery for decortication. Continue Rocephin. Continue Flagyl. . ID evaluation. CT surgery evaluation.
-Oral herpes-due to severity will start acyclovir IV. ID evaluation. Has tested negative for HIV.
-Dilated cardiomyopathy with reduced ejection fraction-difficult to implement goal-directed therapy due to hypotension. Will attempt to introduce carvedilol slowly.
-History of alcohol abuse
.-Remote history of methamphetamine abuse.
-LE PAIN-venous Doppler of lower extremities
-DVT
prophylaxis-subcu enoxaparin
LOS: 5 days
Disposition Plan: TBD code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 01/17/2024 1:44 PM PST

Samuel Ani, DO - 01/16/2024 1:29 PM PST

Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly and was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure and admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

01/15/2024
Patient seen and examined at bedside. Reported pain in the mouth and some lesions on the tongue.
Patient stated that she usually gets the lesion whenever she is sick. Lesion responds to numbing oral spray,
Per patient. Patient continues to require supplemental oxygen improved from 4 L to 2 L today.

01/16/2024
Continues to have pain in the mouth no reported use of some balm provides some relief. Counseled patient against putting in the mouth because of associated chemicals that could have deleterious effects. Reported she will apply to the lips only. Supplemental oxygen 90 continues to improve currently on 1 L from 2 L 1/15/2024. Mild pain in the lower extremity, right. WBC up trended this morning and patient noted to have an episode of fever spike 1/15/24-concerning for persistent infection in spite of antibiotics. CT chest repeated today showing residual fluid in the right pleural space. Evaluated by pulmonology that plans tPA/DNase intrapleural injection for remaining loculations.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 109/74 (BP Location: Right arm, Patient Position: Lying) | Pulse 107 | Temp 37.4 °C (99.3 °F) (Oral) | Resp 20 | Ht 1.702 m (5' 7") | Wt 63.6 kg (140 lb 3.4 oz) | LMP (LMP Unknown) | SpO2 97% | BMI 21.96 kg/m²
I/O: In: 1497.1 [P.O.:840; IV Piggyback:657.1]
Out: 950 [Urine:850; Chest Tube:100]
General-well-developed, well-nourished, drowsy-perhaps due to sedative that she received last night. In
HEENT-normocephalic atraumatic
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. Right chest tube present and functional.
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-normal bulk and tone
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/16/24
0519 | 01/15/24
0528 | 01/14/24
0502 | 01/14/24
0022 | 01/13/24
2028 | 01/13/24
1238 | 01/13/24

0712 | 01/13/24
0514 | 01/12/24
1939 |
WBC	K/mcL	**13.1***	**12.5***	**14.3***	--	--	--	--	**21.4***	**18.7***
HGB	g/dL	**10.6***	**10.4***	**9.7***	--	--	--	--	**11.8***	13.2
TOTAL HGB	g/dL	--	--	--	--	--	--	**11.1***	--	--
MCV	fL	**104***	**102***	97	--	--	--	--	**101***	99
PLT	K/mcL	**546***	**543***	**573***	--	--	--	--	**801***	*
*799***
SODIUM	mmol/L	**131***	**135***	**133***	**134***	**132***	< >	--	**133***	137
SODIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	**134***	--	--
POTASSIUM	mmol/L	4.6	4.2	4.6	4.6	4.7	< >	--	**6.0***	5.1
POTASSIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	**5.3***	--	--
CHLORIDE	mmol/L	100	102	104	104	103	< >	--	101	100
CHLORIDE, WHOLE BLOOD	mmol/L	--	--	--	--	--	--	98	--	--
CO2	mmol/L	**37***	**34***	26	28	27	< >	--	28	**32***
BUN	mg/dL	**20***	**33***	**50***	**53***	**56***	< >	--	**55***	**48***
CREATININE	mg/dL	0.70	0.90	**1.20***	**1.22***	**1.30***	< >	--	**1.39***	**1.09***
CALCIUM	mg/dL	8.5	8.4	**8.1***	**8.3***	**8.0***	< >	--	8.7	9.6
CALCIUM ION	mmol/L	--	--	--	--	--	--	1.17	--	--
MAGNESIUM	mg/dL	--	--	2.3	--	--	--	--	--	--
PHOSPHORUS	mg/dL	--	--	--	2.9	3.5	< >	--	**9.4***	--
GLUCOSE	mg/dL	116	102	**144***	**173***	**135***	< >	--	**235***	**132***
ALBUMIN	g/dL	**2.8***	--	--	**2.6***	**2.7***	< >	--	**3.4***	3.7
AST	Units/L	**102***	--	--	--	--	--	--	--	**70***
ALT	Units/L	19	--	--	--	--	--	--	--	31
BILIRUBIN TOTAL	mg/dL	0.4	--	--	--	--	--	--	--	0.8
ALK PHOS	Units/L	95	--	--	--	--	--	--	--	**133***
TROPONIN I	ng/mL	--	--	--	--	--	--	--	0.014	0.029
CK	Units/L	--	--	128	--	--	--	--	**148***	--
NT PRO-BNP	pg/mL	--	--	--	--	--	--	--	--	**13,700***
LIPASE	Units/L	--	--	--	--	--	--	--	--	114
< > = values in this interval not displayed.

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth to Date
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus

| **AFB Culture w/Stain** |
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Smear	No AFB seen by fluorescent stain
Wound/Fluid/Tissue Culture with stain
	Collect
ion Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth to Date
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)

Vascular Ult venous duplex lower extremity complete bilateral

Result Date: 1/16/2024
IMPRESSION: Negative for deep vein thrombosis in both lower extremities. release

CT chest without contrast for pneumonia STAT

Result Date: 1/16/2024
IMPRESSION: 1. Small to moderate right pleural effusion with indwelling pleural drainage catheter. Subjacent right basilar atelectasis/consolidation 2. Right upper lobe groundglass opacity with peripheral consolidation (reverse halo sign). Findings can be seen in the setting of atypical community-acquired bacterial pneumonia, fungal pneumonia, and organizing pneumonia. release

X-ray portable chest single frontal view

Result Date: 1/16/2024
IMPRESSION: No significant interval change. release CC Dr: p: f:

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. release was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

#Hypoxic respiratory failure
#Sepsis secondary to right-sided empyema with chest tube placement on 1/13
Vancomycin (1/12–1/15), discontinued following negative MRSA surveillance.
-Extubated 1/14
Strep pneumonia antigen positive
Chest CT performed 1/16/2024 showed residual right pleural fluid. Patient evaluated by pulmonology today, Dr. Dammert, who has ordered tPA/DNase
-Thoracentesis with exudative fluid
-pleural fluid culture collected 1/13, no growth to date
-AFB culture with stain performed on 01/13/2024-no AFB stain
-Blood culture, collected 01/12/2024, no growth to date
-Influenza A/B and COVID-19 rapid PCR/NAAT-both negative
-Continue chest tube to waterseal
-Right chest tube in place draining
-Currently on Rocephin, Flagyl ,(1/12-)

-Patient seen by nutritionist who recommended continuation of regular diet, dysphagia 3 diet SLP-patient consumes about 70% of her meal 1/15/2024.

#A-fib RVR
-S/p amnio gtt.
Telemetry monitoring

#HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation
-Holding Coreg and lisinopril given soft BP

#AKI
Creatinine 1.2 from baseline of 0.9.
Normal value 1/15/2024
Continue to avoid nephrotoxic medications

#Hyponatremia
Continue to monitor serum sodium

#Transaminitis
AST of 102
Continue to monitor

#Asthma
-DuoNebs as needed

 # Mouth pain and ulcers
-Likely oral herpes
-Possible oral candidiasis
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely patient is having oral herpes.
Patient also reported benefit with lidocaine spray.
-start IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.
-Start oral clotrimazole.

-Up
dated son,.

Samuel Ani, DO
Internal medicine resident ** **

Attending: Dr. Strauser, Martha Dalia, MD

Cosigned by Martha Strauser, MD at 01/17/2024 1:42 PM PST
Electronically signed by Samuel Ani, DO at 01/16/2024 1:44 PM PST
Electronically signed by Samuel Ani, DO at 01/16/2024 2:45 PM PST
Electronically signed by Samuel Ani, DO at 01/16/2024 5:25 PM PST
Electronically signed by Martha Strauser, MD at 01/17/2024 1:42 PM PST

Pedro Dammert Coronado, MD - 01/16/2024 11:09 AM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista

Chief complain:
Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

History of Present Illness:
This is hospital day 4 for Elizabeth Tina Petruccio a 59 y/o woman admitted with an empyema

Subjective:
C/o mild SOB

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
Scheduled Meds:acetaminophen, 650 mg, oral, Q8H
acyclovir, 400 mg, intraVENOUS, Q8H
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
enoxaparin, 40 mg, subcutaneous, Daily
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID
sodium chloride, 250 mL, intraVENOUS, Once

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• ipratropium-albuteroL, nebulization, Q4H PRN, Ho, Truc Viet, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• zolpidem, oral, At bedtime PRN, Ani, Samuel, MD

Allergies on file: Patient has no known allergies.

Review of Systems:
10 organ review of systems was negative other than what mentioned in HPI

**Family History: **
[table#2]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Social History:
[table#3]:
Social History

[table#4]:
Socioeconomic History
•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	Not on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance and Sexual Activity
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file

| Social History Narrative |
| • | Not on file |

[table#5]:
Social Determinants of Health

[table#6]:
Financial Resource Strain: Not on file
Food Insecurity: Unknown (1/3/2024)
	Food Insecurity
	•	In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Not on file
Transportation Needs: Unknown (1/3/2024)
	Transportation Needs
	•	In
the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Not on file
Physical Activity: Not on file
Stress: Not on file
Social Connections: Not on file
Intimate Partner Violence: Not on file
Housing Stability: Not on file

Physical Exam:
[table#7]:
Vitals:
	01/16/24 1000
BP:	
Pulse:	108
Resp:	
Temp:	
SpO2:	100%

[table#8]:
| Temp: [36.2 °C (97.2 °F)-38.1 °C (100.6 °F)] 37.7 °C (99.9 °F)
Heart Rate: [70-144] 108
Resp: [17-23] 20
BP: (94-132)/(67-87) 125/87
FiO2 (%): [21 %-100 %] 24 % | SpO2: 100 %
FiO2 (%): 24 %
O2 Flow Rate (L/min): 1 L/min | Height: 170.2 cm (5' 7")
Weight: 63.6 kg (140 lb 3.4 oz)
BMI (Calculated): 22 |
| --- | --- | --- |
Cardiac Rhythm: Sinus tachycardia
In: 1497.1 [P.O.:840; IV Piggyback:657.1]
Out: 950 [Urine:850; Chest Tube:100]

Intake/Output Summary (Last 24 hours) at 1/16/2024 1109
Last data filed at 1/16/2024 0416
[table#9]:
Gross per 24 hour
Intake	1195.7 ml
Output	950 ml
Net	**245.7 ml**

[table#10]:
Last 5 weights
	01/12/24 1931	01/13/24 0205
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)

FiO2 (%): [21 %-100 %] 24 %

EXAM:
GEN: awake, in mld distress

HEAD: NC, NT
NECK: supple, without stridor
ENT: moist mucous membranes, oral ulcers
HEART: RRR, no murmur
LUNGS: diminished in the R base, chest tube in place
ABD: soft, NT, ND,
EXT/MSK: no LE edema,
NEURO: awake and alert, moving all 4 ext to command

**Laboratory Data: ** Personally Reviewed

**Imaging: ** Personally Reviewed

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure
2. Right sided empyema
3. Oral ulcers - possibly recurrent aphthous stomatitis vs herpes

Recommendations:
- Chest tube flushed this morning, not much coming out. CT shows small residual pockets
- Keep chest tube to suction
- Will stat intrapleural TPA/DNAse today for remaining loculations
- No need for surgical decortication as the lung re-expanded well
- Continue cefriaxone/flagyl

Pedro Manuel Dammert Coronado, MD, 1/16/2024, 11:09 AM
Pulmonary and Critical Care Medicine
#8086509

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or multiple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system. Patient's risk is high.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Pedro Dammert Coronado, MD at 01/16/2024 11:10 AM PST

Martha Strauser, MD - 01/16/2024 8:42 AM PST
Formatting of this note might be different from the original.
****[image]****

Hospitalist Progress Note

Subjective
[table#1]:
**SUBJECTIVE **
59 female with history of alcohol abuse and remote methamphetamine abuse with 2 drug screen test negative on 2021 and on admission. Has history of cardiomyopathy with reduced ejection fraction of 26%. Admitted with shortness of breath and cough. Positive urine strep antigen. On chest x-ray found to have a right pleural effusion with atelectasis. CT corroborated pleural effusion. Status postthoracentesis with fluid positive for bacteria and compatible with exudate. Has been on Rocephin Flagyl and vancomycin. Of concern as well complaining of herpesvirus flar
e with severe ulcerations in her mouth. She explains to me whenever she is under stress herpesvirus flares and has been under a lot of stress complaining of horrible mouth pain where we can identify multiple diabetic ulcers.
On admission creatinine 1.2 back to 0.9 after hydration.
Chest tube draining 100 cc over the last 24 hours
Objective
[table#2]:
**OBJECTIVE **
Temp: 37.7 °C (99.9 °F)
Heart Rate: 111

Resp: 20
BP: 125/87
SpO2: 94 %
FiO2 (%): 24 %
O2 Flow Rate (L/min): 1 L/min
Temp (24hrs), Avg:37.2 °C (99 °F), Min:36.2 °C (97.2 °F), Max:38.1 °C (100.6 °F)

SpO2: 94 %
FiO2 (%): 24 %
O2 Flow Rate (L/min): 1 L/min
Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND
Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs
[table#3]:
Results from last 7 days
Lab	Units	01/15/24
0528	01/14/24
0502	01/14/24
0022	01/13/24
0514	01/12/24
1939
SODIUM	mmol/L	**135***	**133***	**134***	< >	137
SODIUM, WHOLE BLOOD		--	--	--	< >	--
POTASSIUM	mmol/L	4.2	4.6	4.6	< >	5.1
POTASSIUM, WHOLE BLOOD		--	--	--	< >	--
CHLORIDE	mmol/L	102	104	104	< >	100
CHLORIDE, WHOLE BLOOD		--	--	--	< >	--
CO2	mmol/L	**34***	26	28	< >	**32***
BUN	mg/dL	**33***	**50***	**53***	< >	**48***
CREATININE	mg/dL	0.90	**1.20***	**1.22***	< >	**1.09***
CALCIUM	mg/dL	8.4	**8.1***	**8.3***	< >	9.6
BILIRUBIN TOTAL	mg/dL	--	--	--	--	0.8
ALK PHOS	Units/L	--	--	--	--	**133***
ALT	Units/L	--	--	--	--	31
AST	Units/L	--	--	--	--	**70***
GLUCOSE	mg/dL	102	**144***	**173***	< >	**132***
< > = values in this interval not displayed.

[table#4]:
Results from last 7 days
Lab	Units	01/16/24
0519	01/15/24
0528	01/14/24
0502
WBC	K/mcL	**13.1***	**12.5***	**14.3***
HGB	g/dL	**10.6***	**10.4***	**9.7***
HEMATOCRIT	%	**35.1***	**33.9***	**30.4***
PLT	K/mcL	**546***	**543***	**573***
NEUTROS PCT AUTO	%	**85.4***	**85.7***	**84.2***
LYMPHS PCT AUTO	%	**8.1***	**7.5***	**9.7***
MONOS PCT AUTO	%	**4.1***	**3.8***	**3.0***
EOS PCT AUTO	%	0.2	0.2	0.1

[table#5]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/13/24
0514	01/12/24
1939

| CK | Units/L | 128 | **148*** | -- |
| TROPONIN I | ng/mL | -- | 0.014 | 0.029 |

[table#6]:
Results from last 7 days
Lab	Units	01/12/24
1939
NT PRO-BNP	pg/mL	**13,700***

[table#7]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final
01/14/2024 05:17 PM	93	70 - 125 mg/dL	Final
01/14/2024 12:29 PM	101	70 - 125 mg/dL	Final

[table#8]:
Recent Results (from the past 168 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	Influenza A PCR	Not Detected	Not Detected
	Influenza B PCR	Not Detected	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 P
M
	Specimen: Urine, clean catch
Result	Value	Ref Range
	Urine Culture		
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value	Ref Range
	Blood Culture	No Growth to Date	
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value	Ref Range
	Blood Culture	No Growth to Date	
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value	Ref Range
	Staph Screen culture		
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value	Ref Range
	AFB Smear	No AFB seen by fluorescent stain	
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value	Ref Range
	Wound Culture	No Growth to Date	
	Gram Stain	Many WBCs	
	Gram Stain	No organisms seen	
Respiratory culture with stain

	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value	Ref Range
	Culture	No Growth Final	
	Gram Stain	Moderate WBCs	
	Gram Stain	No organisms seen	
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value	Ref Range
	Strep Pneumo Ag,UR	Positive (A)	Negative

Imaging Studies
X-ray chest single frontal view

Result Date: 1/15/2024
IMPRESSION: 1. Worsening right basilar atelectasis/consolidation. 2. Persistent right pleural effusion. 3. Right chest tube is unchanged. No pneumothorax. release CC Dr: p: f:

Current Scheduled Medications:
acetaminophen, 650 mg, oral, Q8H
acyclovir, 400 mg, intraVENOUS, Q8H
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
heparin (porcine), 5,000 Units, subcutaneous, Q8H SCH
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
multivit-iron-FA-calcium-mins, 1 tablet, oral, Daily
sennosides-docusate sodium, 1 tablet, oral, BID
sodium chloride, 250 mL, intraVENOUS, Once
vancomycin, 750 mg, intraVENOUS, Q12H

Current Infusion Medications:
dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN
bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and lactated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 01/16/24 0312
ipratropium-albuteroL, 3 mL, Q4H PRN
lidocaine, 1 spray, 4x Daily PRN, 1 mL at 01/16/24 0251
melatonin, 10 mg, At bedtime PRN, 10 mg at 01/15/24 0314
ondansetron, 4 mg, Q6H PRN, 4 mg at 01/13/24 0030
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
zolpidem, 5 mg, At bedtime PRN, 5 mg at 01/15/24 2215

Assessment/Plan
[table#9]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with remote history of methamphetamine abuse and alcohol abuse with cardiomyopathy with reduced ejection fraction. Admitted with shortness of breath and pneumonia with right pleural effusion. Status post intubation. Extubated on January 14. Status post thoracentesis with evidence of exudative growing bacteria. Urine strep antigen positive. On Rocephin vancomycin and metronidazole awaiting
final culture and sensitivity results. From the point of view of pneumonia feels better. Complaining of severe oral pain due to multiple oral ulcers. Has been diagnosed with herpes and this is one of her bad flares

-Right lung pneumonia with pleural exudates suggestive of parapneumonic effusion /empyema. Chest tube appears to be draining yellowish fluid. If fluid draining stops may require further interventions including cardiothoracic surgery for decortication. Continue Rocephin. Continue Flagyl. If MRSA screen negative may DC vancomycin. ID evaluation. CTC C and S
-Oral herpes-due to severity will start acyclovir IV. ID evaluation. Has tested negative for HIV.
-Dilated cardiomyopathy with reduced ejection fraction-difficult to implement goal-directed therapy due to hypotension. Will attempt to introduce carvedilol slowly.
-History of alcohol abuse
.-Remote history of methamphetamine abuse.
-LE PAIN-venous Doppler of lower extremities
-DVT prophylaxis-subcu enoxaparin
LOS: 4 days
Disposition Plan: TBD code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 01/16/2024 12:46 PM PST

Jill F - 01/16/2024 8:19 AM PST
Formatting of this note might be different from the original.

[table#1]:
	01/16/24 0740
Oxygen Therapy/Pulse Ox
Oxygen Therapy	Supplemental oxygen
O2 Delivery Method	Nasal cannula
O2 Flow Rate (L/min)	(S) 1 L/min
(On 2 L/min, SpO2=95%. Decreased to 1 L/min.)
FiO2 (%)	24 %
SpO2	94 %
Home Oxygen	N/A
Patient Activity	At rest
$ Pulse Oximetry Spot Check	Yes
Skin Assessment
Skin Assessment	WDL
Vital Signs
Heart Rate	111
Resp	20
Patient Position	Lying
Respiratory Assessment
Respiratory Pattern	Normal
Chest Assessment	Symmetrical
Cough	None
R Upper Breath Sounds	Clear;Diminished
R Lower Breath Sounds	Diminished
L Upper Breath Sounds	Clear;Diminished
L Lower Breath Sounds	Diminished

Electronically signed by Jill F at 01/16/2024 8:19 AM PST

Samuel Ani, DO - 01/15/2024 4:57 PM PST
Formatting of this note might be different from the original.
[image]
Hospital Medicine Progress Note

Subjective

[table#1]:
Interval Events/Subjective

55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. release was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

01/15/24:
Patient seen and examined at bedside. Reported pain in the mouth and some lesions on the tongue.
Patient stated that she usually gets the lesion whenever she is sick. Lesion responds to numbing oral spray,
Per patient. Patient continues to require supplemental oxygen improved from 4 L to 2 L today.

[table#2]:
ROS
A 10 point review of systems was performed and all negative except per HPI.

Objective

[table#3]:
Physical Exam
BP 102/67 (BP Location: Left arm, Patient Position: Lying) | Pulse 104 | Temp 37 °C (98.6 °F) (Oral) | Resp 23 | Ht 1.702 m (5' 7") | Wt 63.6 kg (140 lb 3.4 oz) | LMP (LMP Unknown) | SpO2 97% | BMI 21.96 kg/m^2
I/O: In: 1603.8 [P.O.:670; I.V.:53.8; NG/GT:510; IV Piggyback:370]
Out: 2400 [Urine:2210; Chest Tube:190]
General-well-developed, well-nourished, anxious
HEENT-normocephalic atraumatic
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi with diminished breath sounds on the right side. Right chest tube present and functional.
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-normal bulk and tone
**S
kin**-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#4]:
Data
I have reviewed all of the labs from today. Pertinent labs include:

[table#5]:
Results from last 7 days
Lab	Units	01/15/24
0528	01/14/24
0502	01/14/24
0022	01/13/24
2028	01/13/24
1238	01/13/24
0712	01/13/24
0514	01/12/24
1939
WBC	K/mcL	**12.5***	**14.3***	--	--	--	--	**21.4***	**18.7***
HGB	g/dL	**10.4***	**9.7***	--	--	--	--	**11.8***	13.2
TOTAL HGB	g/dL	--	--	--	--	--	**11.1***	--	--
MCV	fL	**102***	97	--	--	--	--	**101***	99
PLT	K/mcL	**543***	**573***	--	--	--	--	**801***	**799***
SODIUM	mmol/L	**135***	**133***	**134***	**132***	< >	--	**133***	137
SODIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	**134***	--	--
POTASSIUM	mmol/L	4.2	4.6	4.6	4.7	< >	--	**6.0***	5.1
POTASSIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	**5.3***	--	--
CHLORIDE	mmol/L	102	104	104	103	< >	--	101	100
CHLORIDE, WHOLE BLOOD	mmol/L	--	--	--	--	--	98	--	--
CO2	mmol/L	**34***	26	28	27	< >	--	28	**32***
BUN	mg/dL	**33***	**50***	**53***	**56***	< >	--	**55***	**48***
CREATININE	mg/dL	0.90	**1.20***	**1.22***	**1.30***	< >	--	**1.39***	**1.09***
CALCIUM	mg/dL	8.4	**8.1***	**8.3***	**8.0***	< >	--	8.7	9.6
CALCIUM ION	mmol/L	--	--	--	--	--	1.17	--	--
MAGNESIUM	mg/dL	--	2.3	--	--	--	--	--	--
PHOSPHORUS	mg/dL	--	--	2.9	3.5	< >	--	**9.4***	--
GLUCOSE	mg/dL	102	**144***	**173***	**135***	< >	--	**235***	**132***
ALBUMIN	g/dL	--	--	**2.6***	**2.7***	< >	--	**3.4***	3.7

AST	Units/L	--	--	--	--	--	--	--	**70***
ALT	Units/L	--	--	--	--	--	--	--	31
BILIRUBIN TOTAL	mg/dL	--	--	--	--	--	--	--	0.8
ALK PHOS	Units/L	--	--	--	--	--	--	--	**133***
TROPONIN I	ng/mL	--	--	--	--	--	--	0.014	0.029
CK	Units/L	--	128	--	--	--	--	**148***	--
NT PRO-BNP	pg/mL	--	--	--	--	--	--	--	**13,700***
LIPASE	Units/L	--	--	--	--	--	--	--	114
< > = values in this interval not displayed.

[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
**Bloo
d culture**
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth to Date
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Smear	No AFB seen by fluorescent stain
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth to Date
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth to Date
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value

| | Strep Pneumo Ag,UR | Positive (A) |

X-ray chest single frontal view

Result Date: 1/15/2024
IMPRESSION: 1. Worsening right basilar atelectasis/consolidation. 2. Persistent right pleural effusion. 3. Right chest tube is unchanged. No pneumothorax. release CC Dr: p: f:

Assessment/Plan

[table#7]:
Assessment and Plan
55-year-old female with history of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath on 1/12/2024. Patient stated that the symptoms of shortness of breath started 2 days prior to presentation with cough fever dyspnea and chest pain. In the ED x-ray revealed large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. release was found to have empyema and had to be intubated for acute hypoxic and hypercapnic respiratory failure admitted to ICU. Also noted to have A-fib RVR requiring diltiazem gtt. Patient extubated 1/14 and transferred out of the ICU.

#Hypoxic respiratory failure
#Sepsis secondary to right-sided empyema with chest tube placement on 1/13
-Extubated 1/14
Strep pneumonia antigen positive
-Thoracentesis with exudative fluid
-pleural fluid culture collected 1/13, no growth to date
-AFB culture with stain performed on 01/13/2024-no AFB stain
-Blood culture, collected 01/12/2024, no growth to date
-Influenza A/B and COVID-19 rapid PCR/NAAT-both negative
-Continue chest tube to waterseal
-Pulmonary following, appreciate recs
-Right chest tube in place draining
-Currently on Rocephin, Flagyl , Vancomycin (1/12-)
-Patient seen by nutritionist who recommended continuation of regular diet, dysphagia 3 diet SLP

#A-fib RVR
-S/p amnio gtt.
Telemetry monitoring

#HFrEF not in exacerbation
Recent echo ejection fraction 25% with severe LV dilation
-Spot dose Lasix daily as needed to keep net negative
-Holding Coreg and lisinopril given soft BP

#AKI
Creatinine 1.2 from baseline of 0.9.
Normal value 1/15/2024
Continue to avoid nephrotoxic medications

#Asthma
-DuoNebs as needed

 # Mouth pain and ulcers
Patient reports that she gets mouth pain and ulcers whenever she is sick. Examination of the buccal cavity showed lesions on the tongue consistent with herpes infection. It is likely patient is having oral herpes.
Patient also reported benefit with lidocaine spray.
-start IV acyclovir 400 mg (1/15-) 8 hourly for 10 days.

-Updated daughter EriKa.

Samuel Ani, DO
Internal medicine resident ** **

Attending: Dr. St
rauser, Martha Dalia, MD

Cosigned by Martha Strauser, MD at 01/16/2024 12:45 PM PST
Electronically signed by Samuel Ani, DO at 01/15/2024 5:24 PM PST
Electronically signed by Samuel Ani, DO at 01/15/2024 5:31 PM PST
Electronically signed by Samuel Ani, DO at 01/15/2024 5:34 PM PST

Electronically signed by Martha Strauser, MD at 01/16/2024 12:45 PM PST

Martha Strauser, MD - 01/15/2024 1:16 PM PST

Formatting of this note might be different from the original.

****[image]****

Hospitalist Progress Note

Subjective

[table#1]:

**SUBJECTIVE **

59 female with history of alcohol abuse and remote methamphetamine abuse with 2 drug screen test negative on 2021 and on admission. Has history of cardiomyopathy with reduced ejection fraction of 26%. Admitted with shortness of breath and cough. Positive urine strep antigen. On chest x-ray found to have a right pleural effusion with atelectasis. CT corroborated pleural effusion. Status postthoracentesis with fluid positive for bacteria and compatible with exudate. Has been on Rocephin Flagyl and vancomycin. Of concern as well complaining of herpesvirus flare with severe ulcerations in her mouth. She explains to me whenever she is under stress herpesvirus flares and has been under a lot of stress complaining of horrible mouth pain where we can identify multiple diabetic ulcers.

On admission creatinine 1.2 back to 0.9 after hydration.

Objective

[table#2]:

**OBJECTIVE **

Temp: 36.2 °C (97.2 °F)
Heart Rate: 98
Resp: 19
BP: 94/72
SpO2: 97 %
FiO2 (%): 28 %
O2 Flow Rate (L/min): 2 L/min
Temp (24hrs), Avg:36.7 °C (98.1 °F), Min:36.2 °C (97.2 °F), Max:37 °C (98.6 °F)

SpO2: 97 %
FiO2 (%): 28 %
O2 Flow Rate (L/min): 2 L/min

Physical Exam
GEN - normal
HEENT - PERRL
CV - RRR, no RMG
PULM - coarse bilaterally
GI - no organomegaly appreciated, SNTND
Neuro - normal
MSK - MAE
Skin - no rash, no CCE

Labs

[table#3]:

Results from last 7 days
Lab	Units	01/15/24
0528	01/14/24
0502	01/14/24
0022	01/13/24
0514	01/12/24
1939
SODIUM	mmol/L	**135***	**133***	**134***	< >	137
SODIUM, WHOLE BLOOD		--	--	--	< >	--
POTASSIUM	mmol/L	4.2	4.6	4.6	< >	5.1
POTASSIUM, WHOLE BLOOD		--	--	--	< >	--
CHLORIDE	mmol/L	102	104	104	< >	100
CHLORIDE, WHOLE BLOOD		--	--	--	< >	--
CO2	mmol/L	**34***	26	28	< >	**32***
BUN	mg/dL	**33***	**50***	**53***	< >	**48***
CREATININE	mg/dL	0.90	**1.20***	**1.22***	< >	**1.09***
CALCIUM	mg/dL	8.4	**8.1***	**8.3***	< >	9.6
BILIRUBIN TOTAL	mg/dL	--	--	--	--	0.8
ALK PHOS	Units/L	--	--	--	--	**133***
ALT	Units/L	--	--	--	--	31
AST	Units/L	--	--	--	--	**70***

| GLUCOSE | mg/dL | 102 | **144*** | **173*** | < > | **132*** |
| < > = values in this interval not displayed. |

[table#4]:
Results from last 7 days
Lab	Units	01/15/24
0528	01/14/24
0502	01/13/24
0712	01/13/24
0514	01/12/24
1939
WBC	K/mcL	**12.5***	**14.3***	--	**21.4***	**18.7***
HGB	g/dL	**10.4***	**9.7***	--	**11.8***	13.2
TOTAL HGB	g/dL	--	--	**11.1***	--	--
HEMATOCRIT	%	**33.9***	**30.4***	--	38.8	43.7
PLT	K/mcL	**543***	**573***	--	**801***	**799***
NEUTROS PCT AUTO	%	**85.7***	**84.2***	--	--	--
NEUTROS PCT MAN	%	--	--	--	--	**93.0***
LYMPHS PCT AUTO	%	**7.5***	**9.7***	--	--	--
LYMPHO PCT MAN	%	--	--	--	--	**2.0***
MONOS PCT AUTO	%	**3.8***	**3.0***	--	--	--
MONO PCT MAN	%	--	--	--	--	**2.0***
EOS PCT AUTO	%	0.2	0.1	--	--	--
EOSINO PCT MAN	%	--	--	--	--	0.0

[table#5]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/13/24
0514	0
1/12/24
1939
CK	Units/L	128	**148***	--
TROPONIN I	ng/mL	--	0.014	0.029

[table#6]:
Results from last 7 days
Lab	Units	01/12/24
1939
NT PRO-BNP	pg/mL	**13,700***

[table#7]:
Glucose Blood, POC
Date/Time	Value	Ref Range	Status
01/15/2024 05:52 AM	97	70 - 125 mg/dL	Final
01/15/2024 12:15 AM	103	70 - 125 mg/dL	Final
01/14/2024 05:17 PM	93	70 - 125 mg/dL	Final
01/14/2024 12:29 PM	101	70 - 125 mg/dL	Final

[table#8]:
Recent Results (from the past 168 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	Influenza A PCR	Not Detected	Not Detected
	Influenza B PCR	Not Detected	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM

	Specimen: Urine, clean catch
Result	Value	Ref Range
	Urine Culture		
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value	Ref Range
	Blood Culture	No Growth to Date	
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value	Ref Range
	Blood Culture	No Growth to Date	
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value	Ref Range
	Staph Screen culture		
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value	Ref Range
	AFB Smear	No AFB seen by fluorescent stain	
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value	Ref Range
	Wound Culture	No Growth to Date	
	Gram Stain	Many WBCs	
	Gram Stain	No organisms seen	
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value	Ref Range
	Culture	No Growth to Date	
	Gram Stain	Moderate WBCs	
	Gram Stain	No organisms seen	
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value	Ref Range
	Strep Pneumo Ag,UR	Positive (A)	Negative

Imaging Studies
X-ray chest single frontal view

Result Date: 1/15/2024
IMPRESSION: 1. Worsening right basilar atelectasis/consolidation. 2. Persistent right pleural effusion. 3. Right chest tube is unchanged. No pneumothorax. release CC Dr: p: f:

Current Scheduled Medications:
acyclovir, 400 mg, intraVENOUS, Q8H
cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
heparin (porcine), 5,000 Units, subcutaneous, Q8H SCH
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
sennosides-docusate sodium, 1 tablet, oral, BID
vancomycin, 750 mg, intraVENOUS, Q12H

Current Infusion Medications:
dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

Current PRN Medications:
acetaminophen, 650 mg, Q6H PRN
Or
acetaminophen, 650 mg, Q6H PRN

bisaCODYL, 10 mg, Daily PRN
And
lactulose, 20 g, BID PRN
dextrose 10 % (D10W), 40 mL/hr, Continuous PRN
dextrose 5 % and l
actated Ringer's (D5LR), 50 mL/hr, Daily PRN
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr, Continuous PRN
HYDROmorphone, 0.5 mg, Q3H PRN, 0.5 mg at 01/14/24 2356
ipratropium-albuteroL, 3 mL, Q4H PRN
lidocaine, 1 spray, 4x Daily PRN
melatonin, 10 mg, At bedtime PRN, 10 mg at 01/15/24 0314
ondansetron, 4 mg, Q6H PRN, 4 mg at 01/13/24 0030
And
metoCLOPramide, 10 mg, Q6H PRN
And
prochlorperazine, 5 mg, Q6H PRN
zolpidem, 5 mg, At bedtime PRN

Assessment/Plan
[table#9]:
**ASSESSMENT AND PLAN **
Elizabeth Tina Petruccio is a 59 y.o. year old female with remote history of methamphetamine abuse and alcohol abuse with cardiomyopathy with reduced ejection fraction. Admitted with shortness of breath and pneumonia with right pleural effusion. Status post intubation. Extubated on January 14. Status post thoracentesis with evidence of exudative growing bacteria. Urine strep antigen positive. On Rocephin vancomycin and metronidazole awaiting final culture and sensitivity results. From the point of view of pneumonia feels better. Complaining of severe oral pain due to multiple oral ulcers. Has been diagnosed with herpes and this is one of her bad flares
-Right lung pneumonia with pleural exudates suggestive of parapneumonic effusion /empyema. Chest tube appears to be draining yellowish fluid. If fluid draining stops may require further interventions including cardiothoracic surgery for decortication. Continue Rocephin. Continue Flagyl. If MRSA screen negative may DC vancomycin. ID evaluation.
-Oral herpes-due to severity will start acyclovir IV. ID evaluation. Has been tested negative for HIV.
-Dilated cardiomyopathy with reduced ejection fraction-difficult to implement therapy due to hypotension. Will attempt to introduce carvedilol slowly.
-History of alcohol abuse
.-Remote history of methamphetamine abuse.
-DVT prophylaxis-subcu enoxaparin
LOS: 3 days
Disposition Plan: TBD code Status: Full Code.

Martha Dalia Strauser, MD
Hospitalist

Electronically signed by Martha Strauser, MD at 01/15/2024 1:36 PM PST

Formatting of this note might be different from the original.

[table#1]:
	01/15/24 1225
Referral Data
On-Call Referral	No
Referral Source	Physician
Referral Reason	Homeless/Housing
Living Arrangements
Prior Level of Functioning	Independent
Living Arrangements	Alone
Type of Residence	Homeless
Income Information
Income Source	Pension
Drug/Alcohol Abuse
Substance/Alcohol Abuse	Alcohol;Methamphetamine:
Patient reported meth and alcohol use. Patient was vague and provided minimal information regarding use. Patient reported meth and alcohol use are not a concern.
Per chart, UDS negative.
Mental Health
Mental Status	A&OX4
Mental Health	*Patient denied mental health hx and denied SI/HI. *
Mental Health Action Plan	None
Psychosocial
Behavioral Agreement	No
Barriers to Discharge	None identified
Intervention/Plan/Resources	Alcohol Abuse;Homeless;Medi-Cal Resources;Mental Health;Substance Abuse;Transportation;Senior

**Social Work Initial Assessment **
Patient is a 59F with past medical history of heart failure reduced ejection fraction on echo, reported history of asthma unclear if she had PFTs, medical noncompliance, past alcohol and methamphetamine abuse who was brought in from ambulance at the border. Per chart, patient had been visiting her son in Mexico for unclear duration. Normally gets her care in Long Beach California. She reportedly went to a hospital in Mexico but did not agree with the treatment there so was brought to the border where she then called an ambulance. Per records, patient seen by social work in Newport ED on 07/17/2023. Per SW notes, "Per patient likes staying at HB Navigation Center and has been connected to services and resources through them. Per patient gets $500 in SSI and uses for basic needs." New SW consult placed due to homelessness.

**DME*
*: None
**HOME HEALTH/ SKILLED FACILITY HISTORY: **None
**INSURANCE: **MEDI-CAL
**CLINIC/PCP: **None
**PHARMACY: **None
**TRANSPORTATION: **Public transportation.
**SHELTER HISTORY: **HB Navigation Center
**MAILING ADDRESS: **None
**CURRENT PHONE NUMBER: **619-353-4949
**LEGAL ISSUES: **None reported
**MILITARY HISTORY: **None

SW met with the patient at bedside and introduced the SW role. Patient agreeable to interview but difficult to engage. During bedside visit patient reported long hx of homelessness for over 10 years. Patient reported she stays in the Huntington Beach but travels to Mexico frequently. Patient reported her son (Anthony) is homeless in Huntington Beach and will frequently return to stay with him at various homeless shelters. However, patient also provided verbal consent for tx team to contact daughter (emergency contact Erika) as needed regarding discharge planning needs.

Patient reported no income although reported access to SSI benefits. Per patient, has applied for SSI but only approved for EBT. Patient reported discharge plan is to return to Huntington Beach w/son at discharge.

**Intervention/Plan: **
- SW provided supportive counseling, reflective listening and psychoeducation.
- SW provided patient with resources via WDI.
- **SLP Discharge Recommendations:** Independent
- Patient to be offered weather appropriate clothing, meal to go and bus pass at discharge. Bedside RN to assist.
- Plan: No further SW intervention at this time. Please consult if needed.

Electronically signed by Your Scripps Health Team at 01/15/2024 3:45 PM PST

Pedro Dammert Coronado, MD - 01/15/2024 10:47 AM PST
Formatting of this note might be different from the original.
Pulmonary Medicine Attending Progress Note - Scripps Chula Vista

Chief complain:
Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

History of Present Illness:
This is hospital day 3 for Elizabeth Tina Petruccio a 59 y/o woman admitted with an empyema

Subjective:
Is very hungry
C/o mild SOB

Past Medical History:
[table#1]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
Scheduled Meds:cefTRIAXone, 2 g, intraVENOUS, Q24H
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1, 15 mL, mouth/throat, Before meals & at bedtime
heparin (porcine), 5,000 Units, subcutaneous, Q8H SCH
lidocaine-menthol, 1 patch, transdermal, Q24H
metroNIDAZOLE, 500 mg, intraVENOUS, Q12H
pantoprazole, 40 mg, intraVENOUS, Daily
sennosides-docusate sodium, 1 tablet, oral, BID
vancomycin, 750 mg, intraVENOUS, Q12H

Continuous Infusions:dextrose 10 % (D10W), 40 mL/hr
D5 %-0.45 % sodium chloride (D5W 1/2 NS), 75 mL/hr

PRN Meds:.• acetaminophen, oral, Q6H PRN **OR** acetaminophen, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , Until Discontinued **AND** sennosides-docusate sodium, oral, BID **AND** bisaCODYL, rectal, Daily PRN **AND** lactulose, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 10 % (D10W), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR), intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• D5 %-0.45 % sodium chloride (D5W 1/2 NS), intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• HYDROmorphone, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD
• ipratropium-albuteroL, nebulization, Q4H PRN, Ho, Truc Viet, MD
• lidocaine, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO
• melatonin, oral, At bedtime PRN, Sassounian, Nicole, DO
• ondansetron, intraVENOUS, Q6H PRN **AND** metoCLOPramide, intraVENOUS, Q6H PRN **AND** prochlorperazine, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD

Allergies on file: Patient has no known allergies.

Review of Systems:
10 organ review of sy
stems was negative other than what mentioned in HPI

**Family History: **
[table#2]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Social History:
[table#3]:
| **Social History** |

| --- |

[table#4]:
Socioeconomic History
•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	Not on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance and Sexual Activity
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#5]:
Social Determinants of Health

[table#6]:
Financial Resource Strain: Not on file
Food Insecurity: Unknown (1/3/2024)
	Food Insecurity
	•	In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Not on file
Transportation Needs: Unknown (1/3/2024)
	Transportation Needs
	•	In the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Not on file
Physical Activity: Not on file
Stress: Not on file
Social Connections: Not on file
Intimate Partner Violence: Not on file
Housing Stability: Not on file

Physical Exam:
[table#7]:
Vitals:
	01/15/24 0730
BP:	
Pulse:	93
Resp:	18
Temp:	
SpO2:	93%

[table#8]:
| Temp: [36.7 °C (98.1 °F)-37 °C (98.6 °F)] 36.8 °C (98.2 °F)
Heart Rate: [63-96] 93
Resp: [11-42] 18
BP: (97-126)/(69-85) 126/85
FiO2 (%): [28 %-36 %] 28 % | SpO2: 93 %
FiO2 (%): 28 %
O2 Flow Rate (L/min): 2 L/min | Height: 170.2 cm (5' 7")
Weight: 63.6 kg (140 lb 3.4 oz)
BMI (Calculated): 22 |
| --- | --- | --- |
Cardiac Rhythm: Normal sinus rhythm
In: 1603.8 [P.O.:670; I.V.:53.8; NG/GT:510; IV Piggyback:370]
Out: 2400 [Urine:2210; Chest Tube:190]

Intake/Output Summary (Last 24 hours) at 1/15/2024 1048
Last data filed at 1/15/2024 1021
[table#9]:
Gross per 24 hour
Intake	1381.35 ml
Output	2100 ml
Net	**-718.65 ml**

[table#10]:
Last 5 weights
	01/12/24 1931	01/13/24 0205
Weight:	65.1 kg (143 lb 8.3 oz)	63.6 kg (140 lb 3.4 oz)

FiO2 (%): [28 %-36 %] 28 %
PEEP/CPAP (cm H2O): [5 cm H20] 5 cm H20
MAP (cm H2O): [7] 7

EXAM:
GEN: awake, in mld distress
HEAD: NC, NT
NECK: supple, without stridor
ENT: moist mucous membranes, oral ulcers
HEART: RRR, no murmur
LUNGS: diminished in the R base, chest tube in place
ABD: soft, NT, ND,
EXT/MSK: no LE edema,
NEURO: awake and alert, moving all 4 ext to command

**Laboratory Data: ** Personally Reviewed

**Imaging: ** Personally Reviewed

Impression:
59 y.o. y/o female with the following problems:
1. Acute hypoxemic respiratory failure
2. Right sided empyema
3. Oral ulcers - possibly recurrent aphthous stomatitis but other etiologies are possible

Recommendations:
- Chest tube flushed this morning
- Keep chest tube to suction
- Will remove chest tube in 1-12 days
- No need for surgical decortication as the lung re-expanded well
- Continue cefriaxone/flagyl/vanco. Follow cultures and can d/c vanco tomorrow if no MRSA isolated

Pedro Manuel Dammert Coronado, MD, 1/15/2024, 10:48 AM
Pulmonary and Critical Care Medicine
#8086509

E&M coding
I have seen and examined the patient today and by signing above indicate the E & M work was done by me in a single or mult iple interval during a 24 hour period in the patients unit. I have discussed the care with the patient and I have coordinated care with the primary team. I have reviewed available medical records, follow up and consult notes, flow sheets, medication record, orders, chest imaging, and serial sequential labs.

MEDICAL DECISION MAKING: This is a high complexity case affecting the respiratory system. Patient's risk is high.

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Pedro Dammert Coronado, MD at 01/15/2024 10:52 AM PST

Jill F - 01/15/2024 8:30 AM PST

Formatting of this note might be different from the original.

[table#1]:
	01/15/24 0730
Oxygen Therapy/Pulse Ox
Oxygen Therapy	Supplemental oxygen
O2 Delivery Method	Nasal cannula
O2 Flow Rate (L/min)	(S) 2 L/min
(Decreased from 4 L/min.)
FiO2 (%)	28 %
SpO2	93 %
Home Oxygen	N/A
Patient Activity	At rest
$ Pulse Oximetry Spot Check	Yes
Skin Assessment
Skin Assessment	WDL
Vital Signs
Heart Rate	93
Resp	18
Patient Position	Lying
Respiratory Assessment
Respiratory Pattern	Normal
Chest Assessment	Symmetrical
Cough	None
R Upper Breath Sounds	Clear
R Lower Breath Sounds	Diminished
L Upper Breath Sounds	Clear
L Lower Breath Sounds	Diminished

Electronically signed by Jill F at 01/15/2024 8:30 AM PST

Sophie B - 01/15/2024 7:46 AM PST

Formatting of this note might be different from the original.

Pharmacokinetic Consult – Vancomycin Dosing Service
Clinical Pharmacy Note

Elizabeth Tina Petruccio is a 59 y.o. female has been started on vancomycin for pulmonary infection. Target AUC/MIC is 400-600 while maintaining trough < 20.

Relevant clinical data:

[table#1]:
Results from last 7 days
Lab	Units	01/15/24
0528	01/14/24
0502	01/14/24
0022	01/13/24
2028	01/13/24
1533	01/13/24
1238	01/13/24
0514	01/12/24
1939
BUN	mg/dL	**33***	**50***	**53***	**56***	**59***	< >	**55***	**48***
CREATININE	mg/dL	0.90	**1.20***	**1.22***	**1.30***	**1.20***	< >	**1.39***	**1.09***
WBC	K/mcL	**12.5***	**14.3***	--	--	--	--	**21.4***	**18.7***
< > = values in this interval not displayed.

Estimated Creatinine Clearance: 65.5 mL/min (by C-G formula based on SCr of 0.9 mg/dL).

[table#2]:
Vancomycin Level, Random
Date	Value	Ref Range	Status
01/14/2024	16	Not Established mcg/mL	Final
01/13/2024	14	Not Established mcg/mL	Final

[table#3]:

Temp Readings from Last 1 Encounters:
01/15/24	36.8 °C (98.2 °F) (Oral)

[table#4]:
Ht Readings from Last 1 Encounters:
01/12/24	1.702 m (5' 7")

63.6 kg (140 lb 3.4 oz)
I/O last 3 completed shifts:
In: 3709.1 [P.O.:670; I.V.:1220.7; NG/GT:970; IV Piggyback:848.4]
Out: 3020 [Urine:2810; Chest Tube:210]

Microbiology culture results:
[table#5]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date
Blood culture

	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth to Date
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Smear	No AFB seen by fluorescent stain
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth to Date
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen

| **Strep pneumo antigen, urine** |
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)

Current antibiotics:
[table#6]:
Anti-infectives (From admission, onward)

[table#7]:
	Start			Dose/Rate	Route	Frequency	Ordered	Stop
	01/15/24 1000		[table#7.1]	750 mg
180 mL/hr over 90 Minutes	intraVENOUS	Every 12 hours	01/15/24 0746	
	01/13/24 0330		[table#7.2]	500 mg
100 mL/hr over 60 Minutes	intraVENOUS	Every 12 hours	01/13/24 0311	01/20/24 0329
	01/12/24 2201		[table#7.3]	2 g
120 mL/hr	intraVENOUS	Every 24 hours	01/12/24 2200	01/16/24 2214

[table#7.1]:
vancomycin (VANCOCIN) 750 mg in sodium chloride 0.9 % (NS) 250 mL Vial2Bag

[table#7.2]:
metroNIDAZOLE in NaCl (iso-os) (FLAGYL) IVPB 500 mg

[table#7.3]:
cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g
[table#8]:

Assessment
Vancomycin day #3
Current dose: 1000 mg IV q 24 H
Precise PK predicts halflife 9.2 hours and AUIC only 303 on current regimen. Predicts AUIC 455 on 750 mg q12

Renal function: Scr improving, currently 0.9 from a peak of 1.39 Urine output is >0.5 mL/kg/hr
Risk factors for nephrotoxicity: recovering AKI
Risk factors for unpredictable body dynamics: no

Plan
Empirically adjust vancomycin to 750 mg IV q12h. Anticipate either checking trough level or DC vanco within the next couple of days.

Pharmacy will continue to follow the patient's culture results and clinical progress daily and adjust the dose as needed.

Sophie J. Bordson, PharmD, 1/15/2024

Electronically signed by Sophie B at 01/15/2024 7:49 AM PST

Your Scripps Health Team - 01/15/2024 7:18 AM PST
Formatting of this note might be different from the original.
Clinical Nutrition Monitor Note - Reassessment; patient extubated

Recommendations to Provider:
1. Continue Regular diet, Dysphagia 3 per SLP
2. Recommend daily MVT with minerals (RD pended)

**Nutrition Diagnosis (PES statement): **
Inadequate oral Intake* *related to previous mechanical ventilation, now extubated with PO diet as evidenced by no charted PO intake yet ~ *ongoing*

[table#1]:

|
|
| --- |
| [table#1.1] |

[table#1.1]:
Dietary Orders
(From admission, onward)

[table#2]:
|
|
| --- |

[table#3]:
| | [table#3.1]
[table#3.2] |
| --- | --- |

[table#3.1]:
	Start			Ordered
	01/15/24 1048		[table#3.1.1]	01/15/24 1047

[table#3.1.1]:
Combination Diet Regular; Dysphagia 3 Advanced Texture Diet effective now
[table#3.1
.1.1]

[table#3.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Food Consistency:	Dysphagia 3 Advanced Texture	

[table#3.2]:

[table#4]:

Food Allergies: NKFA

Nutritionally significant labs:
[table#5]:
Results from last 7 days
Lab	Units	01/15/24
0528	01/14/24
0502	01/14/24
0022
SODIUM	mmol/L	**135***	**133***	**134***

Nutritionally significant medications:
- heparin (porcine), 5,000 Units, subcutaneous, Q8H SCH
- pantoprazole, 40 mg, intraVENOUS, Daily
- sennosides-docusate sodium, 1 tablet, oral, BID

Skin: Intact

**Anthropometrics: **
Height: 170.2 cm (5' 7"), Weight: 63.6 kg (140 lb 3.4 oz), BMI (Calculated): 22

Weight trend this admission: [x] Loss[ ] Gain [ ] Stable [ ] Requested new weight

Weight change assessment:
[table#6]:
	Date			
	Height	Weight	BMI (Calculated)	Weight Method
1/12/2024	170.2 cm	—	22.5 kg/m2	
		65.1		
1/13/2024		63.6	22 kg/m2	Bed Scale

**Estimated needs: **
Weight Used for Equation Calculations: 63.6 kg (140 lb 3.4 oz), Describe Weight Used: Actual
Total Daily Calorie Estimated Needs: 1600-1900 (25-30 kcal/kg) kcal/day
Total Daily Protein Estimated Needs : 65-75 (1-1.2 g/kg) g/day
Total Daily Fluid Estimated Needs: 1600-1900 mL/day

Nutrition Related Summary since last visit:

- Patient extubated 1/14 and transferred out of ICU; estimated needs reassessed as above
- Visited patient at bedside; was on the phone at time of visit and stated "I'm hungry, please send me food", was unable to obtain diet hx at this visit
- SLP eval this AM; patient cleared for Regular, Dysphagia 3 diet
- No PO intake recorded yet
- Patient is on automatic meal tray protocol
- Last BM 1/15
- Chest tube with 190 mL output x 24 hrs

**Is patient meeting estimating needs?: **[ ] Yes [x] No
Are current nutrition related orders adequate and appropriate?: [x] Yes [ ] No
Change recommended: N/A

**Plan of Care Discussion: **Patient and IDT Rounds

Nutrition Level of Care: High (LC3)

Monitoring/Evaluation:
PO intake, need for ONS, weight trend, BM, BG, diet hx

Goals:
Patient has a source of nutrition by next RD visit ~ *met*
PO intake to meet GT 75% estimated needs ~ *new*

Dakota F., MS, RD
Clinical Nutrition
312-7176

Electronically signed by Your Scripps Health Team at 01/15/2024 12:05 PM PST

Your Scripps Health Team - 01/14/2024 3:06 PM PST

Formatting of this note might be different from the original.

[table#1]:
	01/14/24 1500
Clinical Encounter Type
Visit Date	01/14/24
Visited With	Patient
Routine Visit	Introduction
Consult	Patient care
Emotional/Spiritual Services
Assessment	Elizabeth was not disposed for chaplain visit. I offered a silent prayer for her. Chaplains remain available to offer support as needed.
Services Provided	Ministry of Presence;Prayer
Emotional/Spiritual Outcomes
Outcome	Spiritual Care Services remains available as needed.
Continue Visiting	As needed

Thank you for the opportunity to visit this patient. Please place an "Inpatient consult to Spiritual Care" for future visits as needed. Chaplains remain available.
Esther Diaz, 1/14/2024, 3:06 PM

Electronically signed by Your Scripps Health Team at 01/14/2024 3:06 PM PST

Lucie O - 01/14/2024 12:17 PM PST

Formatting of this note might be different from the original.

Pharmacokinetic Consult – Vancomycin Dosing Service
Initiated on: 1/13
Clinical Pharmacist Note

Elizabeth Tina Petruccio is a 59 y.o. female has been started on vancomycin for pneumonia. AUC/MIC goal is 400-600 mg·hr/L.

Relevant clinical data:
[table#1]:
Anti-infectives (From admission, onward)

[table#2]:
	Start			Dose/Rate	Route	Frequency	Ordered	Stop
	01/14/24 1600		[table#2.1]	1 g
180 mL/hr over 90 Minutes	intraVENOUS	Every 24 hours	01/14/24
1216	
	01/13/24 0900		[table#2.2]		other route	Every 12 hours scheduled	01/13/24 0721	
	01/13/24 0330		[table#2.3]	500 mg
100 mL/hr over 60 Minutes	intraVENOUS	Every 12 hours	01/13/24 0311	01/20/24 0329
	01/12/24 2201		[table#2.4]	2 g
120 mL/hr	intraVENOUS	Every 24 hours	01/12/24 2200	01/16/24 2214

[table#2.1]:
vancomycin (VANCOCIN) 1 g in sodium chloride 0.9 % (NS) 250 mL Vial2Bag

[table#2.2]:
Vancomycin Pharmacy Consult (NOT A MEDICATION ORDER)

[table#2.3]:
metroNIDAZOLE in NaCl (iso-os) (FLAGYL) IVPB 500 mg

[table#2.4]:
cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g
[table#3]:

[table#4]:
| **Ht Readings from Last 1 Encounters:** |

| --- |
| 01/12/24 | 1.702 m (5' 7") |

63.6 kg (140 lb 3.4 oz)

[table#5]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/14/24
0022	01/13/24
2028	01/13/24
1533	01/13/24
1238	01/13/24
0514	01/12/24
1939
CREATININE	mg/dL	**1.20***	**1.22***	**1.30***	**1.20***	< >	**1.39***	**1.09***
BUN	mg/dL	**50***	**53***	**56***	**59***	< >	**55***	**48***
WBC	K/mcL	**14.3***	--	--	--	--	**21.4***	**18.7***
VANCOMYCIN RM	mcg/mL	16	--	--	14	--	--	--
< > = values in this interval not displayed.

Estimated Creatinine Clearance: 49.1 mL/min (A) (by C-G formula based on SCr of 1.2 mg/dL (H)).

Temp (24hrs), Avg:37.2 °C (98.9 °F), Min:37 °C (98.6 °F), Max:37.4 °C (99.3 °F)

I/O last 3 completed shifts:
In: 4067 [I.V.:2215.3; NG/GT:500; IV Piggyback:1351.7]
Out: 2770 [Urine:900; Other:300; Chest Tube:1570]

Microbiology culture results:
[table#6]:
Recent Results (from the past 168 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	Influenza A PCR	Not Detected	Not Detected
	Influenza B PCR	Not Detected	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value	Ref Range
	Urine Culture		
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value	Ref Range
	Blood Culture	No Growth to Date	
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value	Ref Range
	Blood Culture	No Growth to Date	
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value	Ref Range
	Staph Screen culture		
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid

| Result | Value | Ref Range |
| | AFB Smear | No AFB seen by fluorescent stain | |
| **Wound/Fluid/Tissue Culture with stain** |
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value	Ref Range
	Wound Culture	No Growth to Date	
	Gram Stain	Many WBCs	
	Gram Stain	No organisms seen	
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value	Ref Range
	Gram Stain	Moderate WBCs	

| | Gram Stain | No organisms seen | |

MRSA Nares Screen: Negative - High negative predictive value for ruling out MRSA pneumonia

Assessment
Renal function:
Urine output is >0.5 mL/kg/hr and stable
Serum creatinine stable, bumps up and down.

Pharmacokinetics:
Dosing per random levels

Plan
Creatinine has been fairly stable at 1.2-1.3, it is reasonable to schedule a dose. Will schedule 1gm IV q24h; estimated half life is 12 hr and AUC is 398, but based on her random levels I do think she will accumulate to therapeutic on this dose.

Will defer serum sampling at this time pending clinical course.
Pharmacist will continue to follow the patient's culture results and clinical progress daily and adjust the dose as needed.

Lucie A. Owens, PharmD
1/14/2024

Electronically signed by Lucie O at 01/14/2024 12:19 PM PST

Ryan O - 01/14/2024 11:15 AM PST
Summary: Extubation

Formatting of this note might be different from the original.
Patient extubated per MD order. Pt placed on 4L nasal cannula. Tolerating well. No signs of respiratory distress or stridor noted.

Electronically signed by Ryan O at 01/14/2024 11:17 AM PST

Pedro Dammert Coronado, MD - 01/14/2024 8:15 AM PST
Formatting of this note might be different from the original.

[image]

Critical Care Attending Physician Note
1/14/2024 (8:15 AM)
Elizabeth Tina Petruccio
MRN: 810760324
Pt Location: 0ICU/11
** **

History of present illness
Subjective
Sedated and intubated

Objective
[table#1]:
Vitals:
	01/14/24 0730
BP:	96/67
Pulse:	73
Resp:	20

| Temp: | |
| SpO2: | 100% |

GEN: sedated and intubated
HEAD: normocephalic, non-traumatic
ENT: neck supple
EYES: anicteric sclera, PERR
HEART: RRR; no murmurs, rubs, or gallops.
LUNGS: BS diminished R base
GI: soft, non-tender, normoactive BS
EXT: trace LE edema
NEURO: sedated and intubated
SKIN: no rash,

Laboratory Data: Personally Reviewed

Imaging: Personally Reviewed

Assesment:

Patient was seen, evaluated, and examined independently** **by myself, the Attending Physician.

Elizabeth Tina Petruccio is a 59 y.o. female with the following problems:
1. Acute hypoxemic respiratory failure
2. Right sided empyema s/p small bore chest tube 1/14
3. AKI
4. Hyperkalemia
5. HFrEF
6. A fib RVR
7. Asthma

Plan:
1. Lasix x 1 today
2. SAT/SBT today
3. Propofol/fentantyl
4. Ceftriaxone/flagyl/vanco
5. Follow cultures
6. Right sided chest tube to suction
7. SQ heparin/protonix
8. SSI
9. Tube feeds
10. Amiodarone drip stopped
11. No full anticoagulation until critical illness improves

Elizabeth Tina Petruccio is critically ill with central nervous system, pulmonary, cardiovascular, renal, metabolic, gastrointestinal, and endocrine organ systems at risk. The patient requires life and organ supporting interventions that require frequent, personal assessment and manipulation by a critical care provider. Withdrawal of, or failure to initiate these interventions on an urgent basis would likely result in sudden, clinically significant or life-threatening deterioration in the patient's condition which requires the highest level of provider preparedness to intervene urgently as well as frequent reevaluation by providers given high risk decompensation.

**Total critical care time evaluating patient, reviewing records, laboratory data, discussing results with family, engaging consultants, and creating a critical care treatment plan, excluding procedures: 50 minutes

Pedro Manuel Dammert Coronado, MD, 1/14/2024, 8:15 AM
Pulmonary and Critical Care Medicine
#808659

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Pedro Dammert Coronado, MD at 01/14/2024 8:17 AM PST

Ryan O - 01/14/2024 6:41 AM PST

Summary: Respiratory

Formatting of this note might be different from the original.
Patient received intubated with ETT 7.0 secured @ 22cm T/G.

Ventilator Settings:

PRVC 20/400/40%/+5

Electronically signed by Ryan O at 01/14/2024 6:42 AM PST

Ji Y - 01/14/2024 4:49 AM PST

Formatting of this note might be different from the original.
Received pt on the following settings:

[table#1]:
	01/13/24 1809
Vent Information
O2 Delivery Method	Endotracheal tube
Vent Mode	PRVC

[table#2
]:
	01/13/24 1809
Settings
Resp Rate (Set)	20
Vt (Set, mL)	400 mL
PEEP/CPAP (cm H2O)	8 cm H20
Insp Time (sec)	0.76 sec
Insp Rise Time (%)	10 %
Trigger Sensitivity Flow (L/min)	1.61 L/min
P/F Calculation
FiO2 (%)	60 %

Lowered the PEEP to 5 and FiO2 to 40% throughout the night. Pt tolerated the vent well.

ABG results were as followed:
[table#3]:
	Latest Reference Range & Units	01/14/24 03:47
Comment		400/20/5
pH, Arterial	7.35 - 7.45	7.45
pCO2, Arterial	35 - 45 mm Hg	42
pO2, Arterial	83 - 108 mm Hg	**56 (CL)**
HCO3, Arterial	21.0 - 28.0 mmol/L	**29.4 (H)**
O2 Sat, Arterial	95.0 - 98.0 %	**90.1 (L)**
Base Excess/Deficit, Arterial	-2.0 - 3.0 mmol/L	**5.5 (H)**
FIO2, Arterial		35.0
Lactic Acid, Arterial	0.4 - 0.9 mmol/L	**1.0 (H)**
Oxygen Device		Ventilator
Modes		PRVC
(CL): Data is critically low
(H): Data is abnormally high
(L): Data is abnormally low

Electronically signed by Ji Y at 01/14/2024 4:56 AM PST
Electronically signed by Ji Y at 01/14/2024 5:05 AM PST

Christopher Jones, MD - 01/13/2024 11:21 PM PST

Formatting of this note might be different from the original.

[image]

Critical Care Night Attending Note
1/14/2024 (2:22 AM)
Elizabeth Tina Petruccio
MRN: 810760324
Pt Location: 0ICU/11
** **

Patient overnight requiring continued critical care level interventions. Time includes detailed signout of patient's clinical history & days events with daytime ICU team as well as nighttime rounding with Charge nurse, Bedside Nurse, Respiratory therapy. Patient remains sedated but will wake up and follow commands in all extremities and as well as open eyes briefly. When I evaluated her she was sedated nicely getting catheterized

Objective
**Laboratory Data: ** Personally Reviewed, of note creatinine has significant change, calcium continues to downtrend slowly to 4.6, sodium slightly improved to 124, glucose in appropriate range

**Imaging: ** Personally Reviewed, of note chest x-ray with right lower lobe consolidation but significant improvement in right loculated effusion after chest tube insertion, chest significant appropriate position, ET tube in appropriate position

Physical Exam:

Gen: Patient is intubated, sedated
HEENT: nomocephalic atruamatic, pupils 2mm and minimally reactive, ETT in mouth
Neck: supple, midline trachea
Cardiovascular: normal S1-S2 regular rate and rhythm without murmurs, rubs or gallops
Pulmonary: mechanical breath sounds bilaterally improved from yesterday and there was absent breath sounds Majorie right
Abdomen: minimal grimace with palpitation of abdomen, soft nontender nondistended with bowel sounds present all 4 quadrants.
Extremities: no clubbing, cyanosis, or edema.
Neuro: appears comfortable on exam, opens eyes and moves all 4 extremities.
Skin: no discernable skin lesions
Psych: unable to evaluate given intubated status
Chest tube to suction, not tidaling, significant mount of output but only about 30 cc since change of shift

Impression

59 y.o. female with past medical history of heart failure reduced ejection fraction on echo, reported history of asthma unclear if she had PFTs, medical noncompliance, past alcohol and methamphetamine abuse presents with shortness of breath found to have empyema

Plan:
1. Remains thoracentesis lab studies including triglycerides and cholesterol, but given very high white blood cell count likely represents empyema.
2. Broad-spectrum antibiotics, follow-up cultures closely
3. . D5LR to 30 starting tube feeding
4. Anemia expired as patient has been slightly appropriately
5. Decrease PEEP to 5, check a.m. ABG, plan for spontaneous breathing trial in a.m. for possible extubation
6. Continue RASS -2 at this time ventilator synchrony
7. Monitor creatinine closely, as well as urine output
8. Follow-up echocardiogram

Elizabeth Tina Petruccio is critically ill with central nervous system, p
ulmonary, cardiovascular, renal, metabolic, gastrointestinal, and endocrine organ systems at risk. The patient requires life and organ supporting interventions that require frequent, personal assessment and manipulation by a critical care provider. Withdrawal of, or failure to initiate these interventions on an urgent basis would likely result in sudden, clinically significant or life-threatening deterioration in the patient's condition which requires the highest level of provider preparedness to intervene urgently as well as frequent reevaluation by providers given high risk decompensation.

**Total critical care time evaluating patient, reviewing records, laboratory data, to include potentially updating family and consultants as well as creating a critical care treatment plan, excluding procedures: 55 min in addition to time documented previously by the Critical Care Team.

Christopher W Jones, MD
Pulmonary & Critical Care Medicine

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Christopher Jones, MD at 01/14/2024 2:28 AM PST

Merriam A - 01/13/2024 4:22 PM PST
Formatting of this note might be different from the original.
Pharmacokinetic Consult – Vancomycin Dosing Service
Initiated on: **1/13/24, Day 1**

Clinical Pharmacist Note

Elizabeth Tina Petruccio is a 59 y.o. female has been started on vancomycin for pneumonia. AUC/MIC goal is 400-600 mg·hr/L.

Relevant clinical data:
[table#1]:
Anti-infectives (From admission, onward)

[table#2]:
	Start			Dose/Rate	Route	Frequency	Ordered	Stop
	01/13/24 1645		[table#2.1]	1 g
180 mL/hr over 90 Minutes	intraVENOUS	Once	01/13/24 1619	
	01/13/24 0900		[table#2.2]		other route	Every 12 hours scheduled	01/13/24 0721	
	01/13/24 0330		[table#2.3]	500 mg
100 mL/hr over 60 Minutes	intraVENOUS	Every 12 hours	01/13/24 0311	01/20/24 0329
	01/12/24 2201		[table#2.4]	2 g
120 mL/hr	intraVENOUS	Every 24 hours	01/12/24 2200	01/16/24 2214

[table#2.1]:
vancomycin (VANCOCIN) 1 g in sodium chloride 0.9 % (NS) 250 mL Vial2Bag

[table#2.2]:
Vancomycin Pharmacy Consult (NOT A MEDICATION ORDER)

[table#2.3]:
metroNIDAZOLE in NaCl (iso-os) (FLAGYL) IVPB 500 mg

[table#2.4]:
cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g
[table#3]:

[table#4]:
Ht Readings from Last 1 Encounters:
01/12/24	1.702 m (5' 7")

63.6 kg (140 lb 3.4 oz)

[table#5]:
Results from last 7 days
Lab	Units	01/13/24
1533	01/13/24
1238	01/13/24
0514	01/12/24
1939
CREATININE	mg/dL	**1.20***	**1.10***	**1.39***	**1.09***
BUN	mg/dL	**59***	**55***	**55***	**48***
WBC	K/mcL	--	--	**21.4***	**18.7***
VANCOMYCIN RM	mcg/mL	14	--	--	--

Estimated Creatinine Clearance: 49.1 mL/min (A) (by C-G formula based on SCr of 1.2 mg/dL (H)).

Temp (24hrs), Avg:36.8 °C (98.3 °F), Min:36.5 °C (97.7 °F), Max:37.1 °C (98.8 °F)

I/O last 3 completed shifts:
In: 1128.9 [I.V.:255.6; IV Piggyback:873.3]
Out: 300 [Other:300]

Microbiology culture results:
[table#6]:
| **Recent Results (from the past 168 hour(s))** |

| --- |
| **Influenza A/B** |
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	Influenza A PCR	Not Detected	Not Detected
	Influenza B PCR	Not Detected	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result
Value	Ref Range
	COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value	Ref Range
	Gram Stain	Many WBCs	
	Gram Stain	No organisms seen	

MRSA Nares Screen: Pending

Assessment
Renal function:
Serum creatinine trending up

Pharmacokinetics:
Random level about 12.5 hours post loading dose was 14 mcg/mL

[image]

Plan
We will give vancomycin 1000 mg IV x1. Serum creatinine is unstable, therefore will not schedule a maintenance dose. Will check another random level tomorrow morning with SCr and re-dose as needed.

Pharmacist will continue to follow the patient's culture results and clinical progress daily and adjust the dose as needed.

Merriam Almero, PharmD, 1/13/2024, 4:28 PM

Electronically signed by Merriam A at 01/13/2024 4:28 PM PST

Your Scripps Health Team - 01/13/2024 1:01 PM PST

Formatting of this note might be different from the original.
[image]

Case Management Initial Discharge Planning
CMA partially completed via phone with dtr Eicka.

Dx: Pt admitted for SOB and had thoracentesis yesterday. Pt is currently intubated and sedated. Pt is on IV abx, amiodarone, propofol and fentanyl drip.

PLOF: homeless, ind. Dtr said when she last spoke w/pt she was in a homeless shelter in Huntington Beach CA and may have been making her way down to Mexico. Pt does not have any family in CA and dtr is in New Jersey. Per chart review, pt was at a hospital in Newport Beach back in 11/2023.

POC: Dtr Erika Dimarello 609-802-3373
PCP: Unsure
Rx: none
DME: denies
SNF Hx: unsure
HH Hx: unsrue
Tx: public
Funding: Mcal, dtr states pt does not get any income

DCP: TBD. Dtr willing to be decision maker if pt is unable to participate in dcp.

[table#1]:
	01/13/24 1200
**Initial Discharge Planning **
Living Arrangements	Other
(per dtr was last know to be in a shelter in Huntington Beach CA)
Designated Family Caregiver	Documented in Emergency Contacts
Prior Level of Functioning	Independent
Home Care Services	No
Assistive Devices	None
Funding or Insurance Concerns	No
Barriers to Discharge	Yes
Patient expects to be discharged to:	underfunded

Channy RN
Inpatient Care Manager
Scripps Mercy Hospital

Electronically signed by Your Scripps Health Team at 01/13/2024 1:06 PM PST

Your Scripps Health Team - 01/13/2024 9:56 AM PST
Formatting of this note might be different from the original.
**Clinical Nutrition Note **
Initial Assessment

**Recommendations to Provider: **
1. TF Rx Recommendation:** Vital AF 1.2 @ 50 mL/hr **
Calories: 1440 kcal + 200 kcal (propofol) = 1640 kcal (meets 100% of needs)
Protein: 90 g (meets 100% of needs)
Free Water: 973 mL
Volume: 1200 mL/day
Carbohydrates: 133 g
% RDI: 96%
Initiate at 10 mL/hr and increase by 10 mL/hr every 4 hrs until goal is met.
Free water flushes per MD; suggest a minimum of 30 mL q4hr for tube patency.

_____

**Assessment: **

Primary Problem/ Hospital Course: 59 y/o female admitted for acute respiratory failure with hypercapnia
**Nutrition Related PMH: **heart failure reduced ejection fraction on echo, reported history of asthma unclear if she had PFTs, medical noncompliance, past alcohol and methamphetamine abuse

Nutrition Hx/Interview:

Unable to Complete Diet History: Patient intubated

Food Allergies : NKFA per EMR

Weight History:
[table#1]:
	Date			
	Height	Weight	BMI (Calculated)	Weight Method
12/22/2021	170.2 cm	68.7	23.7 kg/m2	Bed Scale
	170.2 cm	74.5	25.7 kg/m2	Bed Scale
12/23/2021		69.2	23.9 kg/m2	Bed Scale

**Nutrition related labs: **
[table#2]:
Lab Results
Component	Value	Date/Time
	NA	134 (L)	01/13/2024 07:12 AM
	K	5.3 (H)	01
/13/2024 07:12 AM
	PHOS	9.4 (H)	01/13/2024 05:14 AM
	BUN	55 (H)	01/13/2024 05:14 AM
	CREATININE	1.39 (H)	01/13/2024 05:14 AM
	GFR	44 (L)	01/13/2024 05:14 AM
	ALBUMIN	3.4 (L)	01/13/2024 05:14 AM
	AST	70 (H)	01/12/2024 07:39 PM
	ALKPHOS	133 (H)	01/12/2024 07:39 PM
POC BG 121-235 mg/dL X 24 hours

**Nutrition related meds: **
• Bowel Regimen
• insulin lispro (HumaLOG) 100 unit/mL injection 2-12 Units
• ondansetron (PF) (ZOFRAN) injection 4 mg **AND** metoCLOPramide (REGLAN) injection 10 mg **AND** prochlorperazine (COMPAZINE) injection 5 mg
• pantoprazole (PROTONIX) injection 40 mg
• propofoL (DIPRIVAN) 10 mg/mL infusion

Skin: intact

**I/O: **
Intake/Output Summary (Last 24 hours) at 1/13/2024 1004
Last data filed at 1/13/2024 0840
[table#3]:
Gross per 24 hour
Intake	1263.86 ml
Output	300 ml
Net	**963.86 ml**

Last BM Date: PTA
GI sx: Hypoactive bowel sounds

[table#4]:
|
|
| --- |
| [table#4.1] |

[table#4.1]:
Dietary Orders
(From admission, onward)

[table#5]:
|
|
| --- |

[table#6]:
| | [table#6.1]
[table#6.2] |
| --- | --- |

[table#6.1]:
	Start			Ordered
	01/12/24 2252		[table#6.1.1]	01/12/24 2255

[table#6.1.1]:
NPO Diet NPO; Sips With Meds Diet effective now
[table#6.1.1.1]

[table#6.1.1.1]:
Question	Answer	Comment
NPO diet:	NPO	
NPO except:	Sips With Meds	

[table#6.2]:

[table#7]:

NFPE Findings: Last completed: 1/13/2024
Subcutaneous Fat Loss:
OrbitalFat Pads[x] None [ ] Mild [ ] Moderate [ ] Severe [ ] UTA
Buccal Fat Pads[x] None [ ] Mild [ ] Moderate [ ] Severe [ ] UTA
Triceps [x] None [ ] Mild [ ] Moderate [ ] Severe [ ] UTA
Midaxillary Line[ ] None [ ] Mild [ ] Moderate [ ] Severe [x] UTA
Ribs[ ] None [ ] Mild [ ] Moderate [ ] Severe [x] UTA
Muscle Loss:
Temple Region[ ] None [x] Mild [ ] Moderate [ ] Severe [ ] UTA
Clavicle Region[x] None [ ] Mild [ ] Moderate [ ] Severe [ ] UTA
Shoulder Region[x] None [ ] Mild [ ] Moderate [ ] Severe [ ] UTA
Scapula Region[ ] None [ ] Mild [ ] Moderate [ ] Severe [x] UTA
Hand Region[x] None [ ] Mild [ ] Moderate [ ] Severe [ ] UTA
Thigh/Patellar Region[ ] None [ ] Mild [ ] Moderate [ ] Severe [x] UTA
Calf Region[x] None [ ] Mild [ ] Moderate [ ] Severe [ ] UTA
Fluid accumulation: none

**Anthropometrics: **
Height: 170.2 cm (5' 7"), Weight: 63.6 kg (140 lb 3.4 oz), BMI (Calculated): 22
IBW: 61 kg

**Weight trend during admission: **
[table#8]:
	Date			
	Height	Weight	BMI (Calculated)	Weight Method
1/12/2024	170.2 cm	65.1	22.5 kg/m2	
		—		
1/13/2024		63.6	22 kg/m2	Bed Scale

**Estimated needs: **
Weight Used for Equation Calculations: 63.6 kg (140 lb 3.4 oz), Describe Weight Used: Actual
Total Daily Calorie Estimated Needs: 1255-1535 (Penn 1394 x 0.9-1.1) kcal/day
Total Daily Protein Estimated Needs : 75-95 (1.2-1.5 gm/kg) gm/day
Total Daily Fluid Estimated Needs: 1590-1910 (25-30 mL/kcal) mL/day

**Nutrition Visit Summary: **
Communication with: unable to speak with patient and RN on unit
*Seen for vent *
- Patient intubated and sedated; no pressors, propofol @ 7.6 mL/hr providing 200 kcal/day. MAP 92.
- NPO orders, TF recommendations above.
- Weight history reviewed.
- Nutrition labs reviewed; multiple derangements.
- Attempted to reach family contacts without success, will continue to follow.

Nutrition Level of Care: High (LC3)

**Nutrition Diagnosis: **
Inadequate oral I
ntake* *related to mechanical ventilation as evidenced by NPO orders.

Interventions:

Nutrition recommendations are provided above (or at the top) and based on the following interventions:
**Food and/or Nutrient Delivery: **
Enteral Nutrition

Coordination of Nutrition Care:
RN and MD

**Monitoring/Evaluation: **
Vent, TF rate/tolerance, meds, electrolytes, diet history

Goals:
Patient has a source of nutrition by next RD visit.

**Sarah P, RD **
Clinical Dietitian
RD Office: 312-7176

Electronically signed by Your Scripps Health Team at 01/13/2024 12:13 PM PST

Pedro Dammert Coronado, MD - 01/13/2024 8:40 AM PST
Formatting of this note might be different from the original.

[image]

Critical Care Attending Physician Note
1/13/2024 (8:40 AM)
Elizabeth Tina Petruccio
MRN: 810760324
Pt Location: 0ICU/11
** **

History of present illness
Subjective
Sedated and intubated

Objective
[table#1]:
Vitals:
	01/13/24 0545
BP:	105/83
Pulse:	84
Resp:	21
Temp:	
SpO2:	94%

GEN: sedated and intubated
HEAD: normocephalic, non-traumatic
ENT: neck supple
EYES: anicteric sclera, PERR
HEART: RRR; no murmurs, rubs, or gallops.
LUNGS: BS diminished R base
GI: soft, non-tender, normoactive BS
EXT: trace LE edema
NEURO: sedated and intubated
SKIN: no rash,

Laboratory Data: Personally Reviewed

Imaging: Personally Reviewed

Assesment:

Patient was seen, evaluated, and examined independently** **by myself, the Attending Physician.

Elizabeth Tina Petruccio is a 59 y.o. female with the following problems:
1. Acute hypoxemic respiratory failure
2. Right sided empyema
3. AKI
4. Hyperkalemia
5. HFrEF
6. A fib RVR
7. Asthma

Plan:
1. Continue current vent settings
2. Propofol/fentantyl
3. Ceftriaxone/flagyl/vanco
4. Right sided chest tube
5. Hyperkalemia improved- trend BMPs
6. SQ heparin/protonix
7. SSI
8. Tube feeds
9. Amiodarone drip
10. No anticoagulation until critical illness improves

Elizabeth Tina Petruccio is critically ill with central nervous system, pulmonary, cardiovascular, renal, metabolic, gastrointestinal, and endocrine organ systems at risk. The patient requires life and organ supporting interventions that require frequent, personal assessment and manipulation by a critical care provider. Withdrawal of, or failure to initiate these interventions on an urgent basis would likely result in sudden, clinically significant or life-threatening deterioration in the patient's condition which requires the highest level of provider preparedness to intervene urgently as well as frequent reevaluation by providers given high risk decompensation.

**Total critical care time evaluating patient, reviewing records, laboratory data, discussing results with family, engaging consultants, and creating a critical care treatment plan, excluding procedures: 50 minutes

Pedro Manuel Dammert Coronado, MD, 1/13/2024, 8:40 AM
Pulmonary and Critical Care Medicine
#808659

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Pedro Dammert Coronado, MD at 01/13/2024 8:49 AM PST
Electronically signed by Pedro Dammert Coronado, MD at 01/13/2024 8:50 AM PST
Electronically signed by Pedro Dammert Coronado, MD at 01/13/2024 1:46 PM PST

Domingo P - 01/13/2024 3:32 AM PST
Formatting of this note might be different from the original.
Placed patient back on NIV due to ABG results. Repeat ABG to follow

Electronically signed by Domingo P at 01/13/2024 3:35 AM PST
Electronically signed by Domingo P at 01/13/2024 3:39 AM PST

Irene G - 01/13/2024 1:29 AM PST
Formatting of this note might be different from the original.
Pharmacokinetic Consult – Vancomycin Dosing Service
Initiated on: 1/13/24
Clinical Pharmacist Note

Elizabeth Tina Petruccio is a 59 y.o. female has been started on vancomycin for possibleempyema. AUC/MIC goal is 400-600 mg·hr/L.

Relevant clinical data:
[table#1]:
Anti-infectives (From admission, onward)

[table#2]:
	Start			Dose/Rate	Route	Frequency	Ordered	Stop
	01/13/24 2000		[table#2.1]	500 mg
270 mL/hr over 60 Minutes	intraVENOUS	Every 24 hours	01/12/24 2200	01/15/24 1959
	01/13/24 1400		[table#2.2]	750 mg
180 mL/hr over 90 M
inutes	intraVENOUS	Every 12 hours	01/13/24 0132	
	01/13/24 0200		[table#2.3]	20 mg/kg × 65.1 kg
125 mL/hr over 120 Minutes	intraVENOUS	Once	01/13/24 0127	
	01/12/24 2201		[table#2.4]	2 g
120 mL/hr	intraVENOUS	Every 24 hours	01/12/24 2200	01/16/24 2214

[table#2.1]:
azithromycin (ZITHROMAX) 500 mg in sodium chloride 0.9 % (NS) 250 mL Vial2Bag

[table#2.2]:
vancomycin (VANCOCIN) 750 mg in sodium chloride 0.9 % (NS) 250 mL Vial2Bag

[table#2.3]:
vancomycin (VANCOCIN) IVPB 1,250 mg in sodium chloride 0.9 % (NS) 250 mL (Premix)

[table#2.4]:
cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g
[table#3]:

[table#4]:
Ht Readings from Last 1 Encounters:
01/12/24	1.702 m (5' 7")

65.1 kg (143 lb 8.3 oz)

[table#5]:
Results from last 7 days
Lab	Units	01/12/24
1939
CREATININE	mg/dL	**1.09***
BUN	mg/dL	**48***
WBC	K/mcL	**18.7***

Estimated Creatinine Clearance: 54 mL/min (A) (by C-G formula based on SCr of 1.09 mg/dL (H)).

Temp (24hrs), Avg:36.6 °C (97.9 °F), Min:36.5 °C (97.7 °F), Max:36.8 °C (98.3 °F)

No intake/output data recorded.

Microbiology culture results:

[table#6]:
Recent Results (from the past 168 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	Influenza A PCR	Not Detected	Not Detected
	Influenza B PCR	Not Detected	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected

MRSA Nares Screen: Pending

Assessment
Renal function:
Urine output not accurately charted and therefore cannot be used to assess drug clearance
Serum creatinine stable

Pharmacokinetics:
Levels not yet obtained

Plan
After receiving a loading dose of 1250 mg IV x1, patient will be initiated on a regimen of 750 mg IV q12h, to which pharmacokinetic modeling predicts a vancomycin half life of 11 hours and an AUC/MIC of 546 mg·hr/L

Will defer serum sampling at this time pending clinical course.
Pharmacist will continue to follow the patient's culture results and clinical progress daily and adjust the dose as needed.

1/13/2024

Electronically signed by Irene G at 01/13/2024 1:34 AM PST

H&P Notes - documented in this encounter

Christopher Jones, MD - 01/12/2024 10:58 PM PST
Formatting of this note might be different from the original.
SCRIPPS CHULA VISTA CRITICAL CARE H&P NOTE

HPI:
Elizabeth Tina Petruccio is a 59 y.o. female with past medical history of heart failure reduced ejection fraction on echo, reported history of asthma unclear if she had PFTs, medical noncompliance, past alcohol and methamphetamine abuse who was brought in from ambulance at the border. History obtained from discussion with ED admitting physician as on my exam patient not waking up sufficiently to corroborate history, does shake her head appropriately though while on BiPAP, knows what hospital and city she is in, and why she was admitted..

Per report, patient has been feeling unwell over the last week with cough fevers shortness of breath chest pain and pressure, had been visiting her son in Mexico for unclear duration. Normally gets her care in Long Beach California. She reportedly went to a hospital in Mexico but did not agree with the treatment there so was brought to the border where she then called an ambulance and was then brought to our hospital here. She was given oxygen for comfort, on admission to the ED she was placed on BiPAP for respiratory distress, as well as poor air movement per report. She was on a BiPAP and diltiazem for A-fib with RVR, given complexity ICU was consulted for admission.

Allergies:
No Known Allergies

Medications:
[table#1]:
**Prior to Admission medications **
Medication	Sig	Start Date	End Date	Taking?	Authorizing Provider
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	**Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310**	**12/23/21**	**1/22/22**		**Arcilla, John-Vincent Lim, MD**
aspirin 81 mg tablet	**Take 1 tablet (81 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310**	**12/24/21**	**1/23/22**		**Arcilla, John-Vincent Lim, MD**
benzonatate (TESSALON) 100 mg capsule	**Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days. Caloptima Mcal ID 93931022D30310**	**12/23/21**	**12/30/21**		**Arcilla, John-Vincent Lim, MD**

carvediloL (COREG) 3.125 mg tablet	**Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day. Caloptima Mcal, ID 93931022D30310**	**12/23/21**	**1/22/22**		**Arcilla, John-Vincent Lim, MD**
furosemide (LASIX) 40 mg tablet	**Take 1 tablet (40 mg total) by mouth daily for 14 days. Caloptima Mcal ID 93931022D30310**	**12/23/21**	**1/6/22**		**Arcilla, John-Vincent Lim, MD**
lisinopriL (PRINIVIL) 10 mg tablet	**Take 1 tablet (10 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310**	**12/24/21**	**1/23/22**		**Arcilla, John-Vincent Lim, MD**
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	**Take 1 tablet by mouth daily. Caloptima Mcal ID 93931022D30310**	**12/23/21**	**1/22/22**		**Arcilla, John-Vincent Lim, MD**

Past Medical History:
[table#2]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Past Surgical History:
No past surgical history on file.

Social History:
[table#3]:
Social History

[table#4]:
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance Use Topics
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently

Family History:
[table#5]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Unable to assess given clinical status

Review of Systems:
A complete review of systems could not be performed due to the clinical status of the patient.

Physical Exam:
**BP 111/85 | Pulse (!) 134 | Temp 36.8 °C (98.3 °F) | Resp 21 | Ht 1.702 m (5'
7") | Wt 65.1 kg (143 lb 8.3 oz) | LMP (LMP Unknown) | SpO2 94% | BMI 22.48 kg/m^2 **
No intake/output data recorded.
FiO2 (%): [60 %] 60 %

EXAM:
GEN: awake, no apparent distress while on BiPAP 12/6
HEAD: NC, NT, punctate white lesions throughout mouth
NECK: supple, without stridor
ENT: moist membranes, clear nares
EYES: PER, anicteric sclera
HEART: RRR, no murmur
LUNGS: Diminished breath sounds right lung throughout, mild left lower lobe diminishment, no obvious wheezes
ABD: soft, mildly tender to palpation, ND, + BS
EXT/MSK: no LE edema, 2+ bil LE pulses
NEURO/PSYCH: awake and alert, moving all 4 extremities spontaneously able to shake her head yes or no to questions appropriately
SKIN: no rashes, no skin lesions
Bedside echo with EF by Simpson at 16%, dilated appearing LV, mild reduction in RV movement with TAPSE 1.3, obvious valvular abnormalities, EPSS elevated at 1.2 no obvious pericardial effusion VTI 17.7 global appearing LV hypokinesis

Database:

[table#6]:
Results from last 7 days
Lab	Units	01/12/24
1939
WBC	K/mcL	**18.7***
HGB	g/dL	13.2
HEMATOCRIT	%	43.7
PLT	K/mcL	**799***

[table#7]:
Results from last 7 days
Lab	Units	01/12/24
1939
SODIUM	mmol/L	137
POTASSIUM	mmol/L	5.1
CHLORIDE	mmol/L	100
CO2	mmol/L	**32***
BUN	mg/dL	**48***
CREATININE	mg/dL	**1.09***
CALCIUM	mg/dL	9.6
ALBUMIN	g/dL	3.7
BILIRUBIN TOTAL	mg/dL	0.8
ALK PHOS	Units/L	**133***
ALT	Units/L	31
AST	Units/L	**70***
GLUCOSE	mg/dL	**132***

[table#8]:
Results from last 7 days
Lab	Units	01/12/24
1939
TROPONIN I	ng/mL	0.029
NT PRO-BNP	pg/mL	**13,700***

[table#9]:
Results from last 7 days
Lab	Units	01/12/24
1939
PROTIME	Seconds	**13.3***
INR		1.1
APTT	Seconds	31

[table#10]:
Results from last 7 days
Lab	Units	01/12/24
1939
TSH	mcIU/mL	1.260

No lab exists for component: "CS30M"

[table#11]:
Results from last 7 days
Lab	Units	01/12/24
1944
PH VEN		**7.26***
PCO2 VEN	mm Hg	**66***
PO2 VEN	mm Hg	28

HCO3 VEN	mmol/L	**29.5***
O2 SAT VEN	%	**41.8***
BASE EXC VEN	mmol/L	2.3

No results found for: "BLOODCULT"

ECG:

Imaging:
X-ray portable chest single frontal view

Result Date: 1/12/2024
IMPRESSION: Large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly. release

Impression:
59 y.o. female with past medical history of heart failure reduced ejection fraction on echo, reported history of asthma unclear if she had PFTs, medical noncompliance, past alcohol and methamphetamine abuse presents with shortness of breath found to have effusion, A-fib with RVR, started on BiPAP for work of breathing

Plan:

1. Urgent thoracentesis, two-physician consent with Dr. Pala, will attempt to wean BiPAP after procedure
2. Continue BiPAP for now, repeat ABG later today, currently has appropriate tidal volume and minute ventilation but will continuously reassess for escalation or de-escalation fusion of therapy
3. Broad-spectrum antibiotics given concern for possible pleural space infection check procalcitonin , blood cultures, thoracentesis cultures
4. Discontinue diltiazem given concerning bedside echocardiogram, start amiodarone for A-fib with RVR, consider heparin drip unless further procedures are needed, formal echo ordered, hold Coreg and lisinopril at this time
5. Repeat EKG, trend troponin given A-fib with RVR
6. Check HIV, give fluconazole for obvious oral thrush given patient on BiPAP
7. Check Legionella and strep antigens
8. Check lower extremity du
plex given asymmetric lower extremity edema
9. N.p.o. status, trend glucose every 6 hours, low-dose dextrose given n.p.o. status and to prevent hypoglycemia

10. **Social**:
-- Daughter called and left a message, will update if she calls back

PROPHYLAXIS:
-GI:
Not indicated

-DVT:
-DVT prophylaxis with heparin

12. CODE STATUS. Full Code

Elizabeth Tina Petruccio is critically ill with central nervous system, pulmonary, cardiovascular, renal, metabolic, and endocrinologic organ systems at risk. The patient requires life and organ supporting interventions that require frequent, personal assessment and manipulation by a critical care physician. Withdrawal of, or failure to initiate these interventions on an urgent basis would likely result in sudden, clinically significant or life-threatening deterioration in the patient's condition which requires the highest level of physician preparedness to intervene urgently as well as frequent reevaluation by providers given high risk decompensation.

**Total critical care time evaluating patient, reviewing records, laboratory data and creating a critical care treatment plan, excluding procedures: 70 min. Since the patient was unable or incompetent to participate in giving a history and/or making treatment decisions, as appropriate, critical care time included discussion with family members or other surrogate decision makers necessary for treatment decisions. The patient requires inpatient critical care services and the patient is expected to spend more than 2 midnights in the intensive care unit.

Christopher Jones MD,
#487454
Pulmonary and Critical Care Medicine

This progress note was created with the use of a Dragon dictation system. The note has been reviewed for correctness and accuracy. Unintended errors may exist due to peculiarities of the dictation system (incorrect words, incorrect spellings and incorrect syntax) despite review and editing.

Electronically signed by Christopher Jones, MD at 01/13/2024 2:16 AM PST

Procedure Notes - documented in this encounter

Christopher Jones, MD - 01/30/2024 1:08 PM PST

Formatting of this note might be different from the original.

Realtime Thoracic Ultrasound of elizabeth Petruccio

Done for diagnostic purposes to evaluate for Fluid
Patient was positioned upright, door and privacy current were closed, exposure of right hemithorax was undertaken. Real-time ultrasound was used to evaluate the area from the anterior chest to the mid axillary line to the posterior position. There were findings similar to patient's thoracentesis 1 week ago, showing consolidated fibrous materials with only a very thin layer of fluid. End of last week's procedure there was some minor bleeding that occurred, I believe that the material noted on ultrasound and on the patient's chest x-rays is likely resolving infectious milieu and possible mild blood clots which will gradually resolve over time. Additionally, there is not a sufficient volume of fluid presents that would make a thoracentesis procedure worth the risk of the procedure. Recommend repeat chest x-ray in a week or so to ensure no worsened accumulation of fluid, but it appears that patient's infection is is under well-controlled and hemithorax has been drained appropriately.

Images from below are from the anterior, lateral, and posterior right hemithorax

[image]

[image]

[image]

[image]

Electronically signed by Christopher Jones, MD at 01/30/2024 1:13 PM PST

Brandon May, DNP - 01/13/2024 11:42 AM PST

Associated Order(s): Chest Tube Insertion
Post-Procedure Diagnose(s): Loculated pleural effusion

Formatting of this note might be different from the original.

[image]
01/13/24 11:42 AM Elizabeth Tina Petruccio MRN: 810760324

Location: 0ICU/11

[table#1]:
| **Critical Care Medicine - Advanced Practice Provider - Pre-Procedure Note **
Scripps Mercy Hospital - Chula Vista |
| --- |

Procedure: Chest Tube

Indication: Empyema

**Discussion with: **
Patient [ ] / Family [x] / Designated Health Care Decision Maker [ ]
Emergency Situation - Unable to obtain consent prior to the procedure [ ]

Benefits of procedure were discussed in detail with the patient/surrogate.

Risks described include: Pneumothorax, Incomplete drainage, Infection, Bleeding, Intercostal nerve damage, Pain.

Alternatives to the procedure include: Not proceeding. Adequate time has been provided for any questions, and I have answered the questions to the best of my abilities.

The Patient [ ] / Family [x] / Designated Health Care Decision Maker [ ] has/have agreed to proceed with the procedure.
Verbal consent has been obtained. Written consent was obtained prior to the procedure.

Code Status: Full Code

[table#2]:
Critical Care Medicine APP Procedure Note

Chest Tube Insertion

Date/Time: **1/13/2024 11:44 AM**

Performed by:** May, Brandon Michael, DNP**
Authorized by: **May, Brandon Michael, DNP**
Consent:
Consent obtained: **Written**
Consent given by: **Healthcare agent**
Risks, benefits, and alternatives were discussed: **yes**
Risks discussed: **Bleeding, damage to surrounding structures, nerve damage, pain, infection and incomplete drainage**
Alternatives discussed: **No treatment**
Universal protocol:
Procedure explained and questions answered to patient or proxy's satisfaction: **yes**
Relevant documents present and verified: **yes**
Test results available: **yes**
Imaging studies available: **yes**
Required blood products, implants, devices, and special equipment available: **yes**
Site/side marked: **yes**
Immediately prior to procedure, a time out was called: **yes**
Patient identity confirmed: **Hospital-assigned identification number and arm band**
Pre-procedure details:
Skin preparation: **Chlorhexidine with alcohol**
Preparation: **Patient was prepped and draped in the usual sterile fashion**
Anesthesia:
Anesthesia method: **Local infiltration**
Local anesthetic: **Lidocaine 1% w/o epi**
Procedure details:
Placement location: **R lateral**
Tube size (French): **14**.
Ultrasound guidance: **yes**
Tension pneumothorax: **no**
Tube connected to: **Water seal**
Drainage characteristics: **Purulent and yellow**
Suture material: **2-0 silk**
Dressi
ng: **Tegaderm**.
Post-procedure details:
Procedure completion: **Tolerated well, no immediate complications**
Comments:
CXR pending, Dr. Dammert at bedside for the entire procedure

**Est. Blood Loss for entire procedure: <10mL
**Sterile Technique: All participants in the procedure were wearing appropriate PPE including: Mask, Cap, Sterile Gloves, Protective Eyewear, and Sterile Gown

**Case was discussed in detail w/ Critical Care Attending, Dr. Dammert

Brandon Michael May, DNP
01/13/24

Cosigned by Pedro Dammert Coronado, MD at 01/13/2024 11:57 AM PST
Electronically signed by Brandon May, DNP at 01/13/2024 11:46 AM PST
Electronically signed by Pedro Dammert Coronado, MD at 01/13/2024 11:57 AM PST

Associated attestation - Pedro Dammert Coronado, MD - 01/13/2024 11:57 AM PST
Formatting of this note might be different from the original.
I was the supervising physician in the delivery of the service.

Christopher Jones, MD - 01/13/2024 5:51 AM PST

Formatting of this note might be different from the original.
Procedure: Endotracheal intubation

Preoperative Diagnoses:
Acute hypoxemic and hypercarbic respiratory failure

Procedure in Detail:
Informed consent could not be obtained due to the emergent nature of the procedure. The patient was preoxygenated with 100% FiO2. Induction was achieved with the administration of etomidate followed by paralysis with rocuronium. The GlideScope was inserted into the mouth, and a grade I view was achieved. Using direct visualization, a size 7.0 endotracheal tube was passed between the cords. The stylet was removed and the tube was advanced to 23 centimeters. The balloon was inflated. Bilateral breath sounds were appreciated. The capnograph turned yellow, consistent with airway placement.

There were no immediate complications. Follow-up chest radiograph is pending at this time.

**EBL: **<5cc

**ASA Score: **4 (severe systemic disease that is a constant threat to life)

Mallanpati Score: 2

Electronically signed by Christopher Jones, MD at 01/13/2024 5:53 AM PST

Christopher Jones, MD - 01/13/2024 1:54 AM PST
Formatting of this note might be different from the original.
Procedure: Diagnostic and therapeutic paracentesis/thoracentesis.

Preoperative Diagnoses:
Pleural effusion

Postoperative Diagnoses:
Possible empyema

Procedure in Detail:
Informed consent could not be obtained due to altered mental status and inability to reach family, however given critical nature of patient I discussed with ED attending Dr. Pala and we are both in agreement to proceed with emergent thoracentesis to help guide treatment. The right hemithorax was examined under ultrasound, and a large pocket of fluid was identified. This area was prepared with chlorhexidine and draped in a sterile fashion. All operators used sterile technique including sterile gloves. Analgesia was obtained using the infiltration of 1% lidocaine into the subcutaneous tissue. A small nick was made with the scalpel. Next, a 5 French centesis catheter needle was inserted into the hemithorax and purulent fluid was obtained. The catheter was advanced and the needle was removed.

A total of 325 of fluid was removed from the patient. Following this, the needle was removed in standard fashion, and a bandage was applied.

The patient tolerated the procedure well. There were no immediate complications. The fluid was sent for appropriate studies.

EBL: Less than 5 cc

Images show fluid that was obtained including chest tubes showing a significant amount of settled sediment, ultrasound image of the procedure area also shown

[image]

[image]

Electronically signed by Christopher Jones, MD at 01/13/2024 1:57 AM PST

Consult Notes - documented in this encounter

Your Scripps Health Team - 02/03/2024 1:42 PM PST
Associated Order(s): IP CONSULT TO NUTRITION SERVICES
Formatting of this note might be different from the original.
**Clinical Nutrition Note **
Brief Consult Note

**Recommendations to Provider: **
1. Continue Regular diet. 1500 mL fluid restriction per provider.
2. Continue multivitamin with minerals daily.

_____

**Assessment: **
Nutrition Services consult received for malnutrition assessment, appreciate consult. Clinical Nutrition currently following patient, please refer to nutrition note from 2/1 for additional details. EMR reviewed.

Food Allergies: lactose intolerance

Skin: Intact

[table#1]:
|
|
| --- |
| [table#1.1] |

[table#1.1]:
Dietary Orders
(From admission, onward)

[table#2]:
|
|
| --- |

[table#3]:
| | [table#3.1]
[table#3.2] |
| --- | --- |

[table#3.1]:
	Start			Ordered
	01/22/24 1101		[table#3.1.1]	01/22/24 1100

[table#3.1.1]:
Combination Diet Regular; 1500 mL; Dental Soft Diet effective now
[table#3.1.1.1]

[table#3.1.1.1]:
Question	Answer	Comment
Please Select Base Diet:	Regular	
Fluid Restriction - Oral (mL/day):	1500 mL	
Food Consistency:	Dental Soft	

[table#3.2]:

[table#4]:

**Estimated needs: **
Weight Used for Equation Calculations: 63.6 kg (140 lb 3.4 oz), Describe Weight Used: Actual
Total Daily Calorie Estimated Needs: 1600-1900 (25-30 kcal/kg) kcal/day
Total Daily Protein Estimated Needs : 65-75 (1-1.2 g/kg) g/day
Total Daily Fluid Estimated Needs: 1600-1900 mL/day

Coordination of Nutrition Care:
N/A

Clinical Nutrition will continue to follow.

Dakota F., MS, RD
Clinical Nutrition
312-7176

Electronically signed by Your Scripps Health Team at 02/03/2024 1:47 PM PST

Samantha Hauff, MD - 01/26/2024 8:09 PM PST

Associated Order(s): IP CONSULT TO ENT

Formatting of this note might be different from the original.

Otolaryngology- Head and Neck Surgery Consultation Note

REFERRING PHYSICIAN: Khayyat, Omar Salim Peter, MD

REASON FOR REFERRAL:

[table#1]:

Chief Complaint
Patient presents with
•	Shortness of Breath
		Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB

HISTORY OF PRESENT ILLNESS: 59F admitted 2 weeks ago for sepsis/empyema. Recently had mild sore throat/neck pain and imaging revealed c-spine infection. She reports she has also had herpes lesions in her oral cavity that are improving. She denies prior surgery on her neck or throat. She denies otalgia or otorrhea. She is worried that she has done damage to her nose by using intranasal drugs in the past.

PAST MEDICAL HISTORY:

[table#2]:

Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

PAST SURGICAL HISTORY: No past surgical history on file.

CURRENT MEDICATIONS:

[table#3]:

Current Facility-Administered Medications
Medication	Dose	Route	Frequency	Provider	Last Rate	Last Admin
•	acetaminophen (TYLENOL) tablet 650 mg	650 mg	oral	Q6H PRN	Dammert Coronado, Pedro Manuel, MD		
	Or
•	acetaminophen (TYLENOL) suppository 650 mg	650 mg	rectal	Q6H PRN	Dammert Coronado, Pedro Manuel, MD		
•	benzonatate (TESSALON) capsule 100 mg	100 mg	oral	TID PRN	Dao, Diep Ngoc, DO		100 mg at 01/26/24 1853
•	sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet	1 tablet	oral	BID	Dammert Coronado, Pedro Manuel, MD		1 tablet at 01/26/24 0925
	And
•	bisaCODYL (DULCOLAX) suppository 10 mg	10 mg	rectal	Daily PRN	Dammert Coronado, Pedro Manuel, MD		
	And
•	lactulose (ENULOSE) 20 gram/30 mL solution 20 g	20 g	oral	BID PRN	Dammert Coronado, Pedro Manuel, MD		
•	calcium carbonate (TUMS) chewable tablet 500 mg	500 mg	oral	BID PRN	Chen, Yu-Chun, DO		500 mg at 01/22/24 1001
•	cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g	2 g	intraVENOUS	Q24H	Khayyat, Omar Salim Peter, MD		2 g at 01/25/24 2147
•	dextrose 10 % (D10W) infusion	40 mL/hr	intraVENOUS	Continuous PRN	Dammert Coronado, Pedro Manuel, MD		
•	dextrose 5 % and lactated Ringer's (D5LR) infusion	50 mL/hr	intraVENOUS	Daily PRN	Dammert Coronado, Pedro Manuel, MD		
•	dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion	75 mL/hr	intraVENOUS	Continuous PRN	Dammert Coronado, Pedro Manuel, MD		
•	diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL	15 mL	mouth/throat	Before meals & at bedtime	Dammert Coronado, Pedro Manuel, MD		15 mL at 01/26/24 1651
•	[Held by provider] enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg	40 mg	subcutaneous	Daily	Ani, Samuel, MD		40 mg at 01/24/24 0855
•	[Held by provider] furosemide (LASIX) tablet 20 mg	20 mg	oral	Daily	Dammert Coronado, Pedro Manuel, MD		20 mg at 01/21/24 0807
•	HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg	0.5 mg	intraVENOUS	Q3H PRN	Dammert Coronado, Pedro Manuel, MD		0.5 mg at 01/25/24 2059
•	hydrOXYzine pamoate (VISTARIL) capsule 25 mg	25 mg	oral	TID PRN	Chen, Yu-Chun, DO		25 mg at 01/26/24 1850
•	ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL	3 mL	nebulization	Q4H PRN	Pena, Jose Rafael, MD		3 mL at 01/26/24 1843
•	lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL	1 spray	mucous membrane	4x Daily PRN	Umukoro, Emuejevuoke Anastasia, DO		1 mL at 01/23/24 2024

•	lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch	1 patch	transdermal	Q24H	Umukoro, Emuejevuoke Anastasia, DO		1 patch at 01/26/24 0309
•	lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch	1 patch	transdermal	Q24H	Piatetsky, Yana, MD		1 patch at 01/26/24 0310
•	melatonin tablet 10 mg	10 mg	oral	At bedtime PRN	Sassounian, Nicole, DO		10 mg at 01/24/24 2104
•	ondansetron (PF) (ZOFRAN) injection 4 mg	4 mg	intraVENOUS	Q6H PRN	Dammert Coronado, Pedro Manuel, MD		4 mg at 01/13/24 0030
	And
•	metoCLOPramide (REGLAN) injection 10 mg	10 mg	intraVENOUS	Q6H PRN	Dammert Coronado, Pedro Manuel, MD		
	And
•	prochlorperazine (COMPAZINE) injection 5 mg	5 mg	intraVENOUS	Q6H PRN	Dammert Coronado, Pedro Manuel, MD		
•	metroNIDAZOLE (FLAGYL) tablet 500 mg	500 mg	oral	Q12H SCH	Shao, Hai, MD, PhD		
•	multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet	1 tablet	oral	Daily	Dammert Coronado, Pedro Manuel, MD		1 tablet at 01/26/24 0925
•	naproxen (NAPROSYN) tablet 500 mg	500 mg	oral	BID PRN	Ani, Samuel, MD		500 mg at 01/26/24 0052
•	oxyCODONE (ROXICODONE) immediate release tablet 5 mg	5 mg	oral	Q4H PRN	Dammert Coronado, Pedro Manuel, MD		5 mg at 01/26/24 1834
•	zolpidem (AMBIEN) tablet 5 mg	5 mg	oral	At bedtime PRN	Ani, Samuel, MD		5 mg at 01/25/24 2144

ALLERGIES:
[table#4]:
Allergies
Allergen	Reactions
•	Lactose Intolerant [Lactose]	Other (see comments)
		Patient having diarrhea; only fluid milk - all other lactose items ok

FAMILY HISTORY:
[table#5]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

SOCIAL HISTORY:
[table#6]:
Social History

[table#7]:
Socioeconomic History
•	Marital status:	Single
		Spouse name:	Not on file
•	Number of children:	No
t on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance and Sexual Activity
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#8]:
Social Determinants of Health

[table#9]:
Financial Resource Strain: Not on file
Food Insecurity: At Risk (1/19/2024)

| | Food Insecurity |
| | • | In the past 3 months, have you had to go without food for 24 hours, multiple times due to lack of resources?: Yes |
| Transportation Needs: At Risk (1/19/2024) |
| | Transportation Needs |
| | • | In the past 3 months, has lack of transporation kept you from medical appointments or getting things you need that are essential to your health?: Yes |
| Physical Activity: Not on file |
| Stress: Not on file |
| Social Connections: Not on file |
| Intimate Partner Violence: Not on file |
| Housing Stability: High Risk (1/19/2024) |
| | Housing Stability |
| | • | What is your living situation today?: Do not have a stable place to live |

ROS:
Review of Systems - ENT ROS: negative except as noted in HPI

VITAL SIGNS: BP 113/82 (Patient Position: Sitting) | Pulse 110 | Temp 36.7 °C (98.1 °F) (Axillary) | Resp 19 | Ht 1.676 m (5' 6") | Wt 68 kg (149 lb 14.6 oz) | LMP (LMP Unknown) | SpO2 96% | Breastfeeding No | BMI 24.20 kg/m^2

GENERAL: WN/WD, NAD
HEAD: NC/AT
EARS: Auricles normal. No post-auricular tenderness.
NOSE: Dorsum midline, no epistaxis or crusts. Anterior rhinoscopy reveals non-obstructed septum, normal turbinates, healthy mucosa.
ORAL CAVITY/OROPHARYNX: No masses or lesions, tonsils 1+. Poor dentition, mild angular cheilitis.
NECK: Neck flat, trachea midline, no LAD. Full neck mobility without pain.
NEUROLOGIC: Cranial nerves II-XII are grossly intact.
PULMONARY: Breathing comfortably on room air.

Labs: WBC 5.7

Imaging: MRI c-spine 1.26.2024
"Impression:

Contrast imaging is very limited due to extensive motion.

Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema.

Findings are concerning for discitis/osteomyelitis at C4-C5."

IMPRESSION: 59F with cervical spine osteomyelitis. ENT exam is within normal limits without any sign of mucosal abnormality or infection. Suspect that fluid is related to c-spine disease process.

PLAN:
-if further concerns, please consider CT neck with contrast

If any questions/concerns, please call 619-320-5595.

Samantha Jill Hauff, MD
1/26/2024 8:09 PM

Electronically signed by Samantha Hauff, MD at 01/26/2024 8:17 PM PST

Hai Shao, MD, PhD - 01/26/2024 2:59 PM PST
Formatting of this note might be different from the original.
CONSULTATION

CONSULTATION REQUESTED BY: Dr. Omar Khayyat
DATE OF SERVICE: 1/26/2024
REASON FOR CONSULT: Antibiotic management on this patient who is admitted to the hospital with acute respiratory failure secondary to severe pulmonary infection now found to have cervical spine discitis and osteomyelitis.

HISTORY OF PRESENT ILLNESS
Elizabeth Tina Petruccio is a 59 y.o. female with past medical history significant for longstanding alcohol and methamphetamine abuse, nonischemic cardiomyopathy and chronic homelessness who was admitted to the hospital 14 days ago with several days of worsening shortness of breath associated with fever and cough. She also complained about multiple ulcerations inside her mouth with odynophagia, attributed to recently diagnosed herpes infection after she was giving oral sex to a person with genital herpes.

The patient was found to be afebrile with severe leukocytosis of 19,000 and thrombocytosis of 800. Procalcitonin level was mildly elevated and her ABG revealed evidence of severe hypercapnic and hypoxic respiratory
failure. Chest x-ray revealed a large right-sided pleural effusion with right base consolidation. The patient was started on IV vancomycin, Rocephin and Flagyl empirically. She was managed initially on BiPAP for respiratory failure with no success. The patient was admitted to the intensive care unit and intubated on hospital day #1. Large volume thoracentesis was performed and the fluid appeared cloudy with significant amount of sediments. Cell counts show significant pleocytosis with elevated LDH and protein consistent with exudative pleural fluid likely empyema. Chest tube was placed afterwards. The patient continued on above antibiotics till sepsis workup came back positive for urine antigen of Streptococcus pneumonia, and negative MRSA screening test and negative culture of the pleural fluid. The patient also received acyclovir for suspected oral herpes infection. She was successfully extubated on hospital day #2 and continued on antibiotic treatment and chest tube drainage till 4 days ago when the chest tube was removed and antibiotic tapered to oral Augmentin. Since then the patient remained afebrile with no leukocytosis. For the past 2 days the patient woke up with severe headache and complained about mild sore throat. MRI of the cervical spine showed large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema. Findings are concerning for discitis/osteomyelitis at C4-C5. IV vancomycin and Rocephin were restarted in place of Augmentin. Infectious disease consultation service was requested for antibiotic management in this regard. Currently the patient is sitting comfortably in bed, dictating a lengthy email by voice. She denied any significant pain inside the mouth, neck stiffness or shortness of breath. The patient reported being homeless recently drinking alcohol on a daily basis. She had multiple episodes of nausea and vomiting associated with diarrhea for months prior to admission. She reported being tested negative for syphilis prior to admission after her exposure with a sexual partner with genital herpes.

ALLERGIES
[table#1]:
Allergies
Allergen	Reactions
•	Lactose Intolerant [Lactose]	Other (see comments)
		Patient having diarrhea; only fluid milk - all other lactose items ok

REVIEW OF SYSTEMS
Review of Systems
Constitutional: Negative for chills, fatigue, fever and unexpected weight change.
HENT: Negative for congestion, sneezing, sore throat, tinnitus, trouble swallowing and voice change.
Eyes: Negative for photophobia and visual disturbance.
Respiratory: Negative for apnea, cough, choking, chest tightness, shortness of breath, wheezing and stridor.
Cardiovascular: Negative for chest pain, palpitations and leg swelling.
Gastrointestinal: Negative for abdominal distention, abdominal pain, constipation, diarrhea, nausea, rectal pain and vomiting.
Endocrine: Negative for polyphagia and polyuria.
Genitourinary: Negative for decreased urine volume, difficulty urinating, dyspareunia, dysuria, flank pain, vaginal bleeding, vaginal discharge and vaginal pain.
Musculoskeletal: Negative for arthralgias, back pain, gait problem, joint swelling, myalgias, neck pain and neck stiffness.
Skin: Negative for color change, pallor, rash and wound.
Neurological: Negative for syncope and weakness.
Hematological: Negative for adenopathy. Does not bruise/bleed easily.
Psychiatric/Behavioral: Negative for agitation, behavioral problems, confusion, decreased concentration and dysphoric mood.

CURRENT MEDICATIONS

Current Facility-Administered Medications:
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/22/24 0846
• Bowe
l Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/26/24 0925 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• calcium carbonate (TUMS) chewable tablet 500 mg, 500 mg, oral, BID PRN, Chen, Yu-Chun, DO, 500 mg at 01/22/24 1001
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Khayyat, Omar Salim Peter, MD, 2 g at 01/25/24 2147
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtime, Dammert Coronado, Pedro Manuel, MD, 15 mL at 01/26/24 1323
• [Held by provider] enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/24/24 0855
• [Held by provider] furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/21/24 0807
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado,

Pedro Manuel, MD, 0.5 mg at 01/25/24 2059
• hydrOXYzine pamoate (VISTARIL) capsule 25 mg, 25 mg, oral, TID PRN, Chen, Yu-Chun, DO, 25 mg at 01/23/24 0021
• ipratropium-albuteroL (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/26/24 0900
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/23/24 2024
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/26/24 0309
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Piatetsky, Yana, MD, 1 patch at 01/26/24 0310
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/24/24 2104
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/13/24 0030 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/26/24 0925
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD, 500 mg at 01/26/24 0052
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/26/24 1323
• vancomycin (VANCOCIN) 750 mg in sodium chloride 0.9 % (NS) 250 mL Vial2Bag, 750 mg, intraVENOUS, Q8H, Giang, Irene, PharmD, Last Rate: 180 mL/hr at 01/26/24 0925, 750 mg at 01/26/24 0925
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/25/24 2144

PAST MEDICAL HISTORY
[table#2]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

FAMILY HISTORY
Positive for hypertension and diabetes.

SOCIAL HISTORY
Elizabeth's reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently us
e drugs.

PHYSICAL EXAM
VITAL SIGNS
[table#3]:
[table#3.1]
Physical Exam

[table#3.1]:
Vitals:
	01/26/24 0725	01/26/24 0909	01/26/24 1135	01/26/24 1200
BP:	120/89		**(!) 136/91**	123/85
BP Location:	Left arm		Left arm		
Patient Position:	Lying		Sitting		
Pulse:	101		117	99
Resp:	17		23	16
Temp:	36.7 °C (98.1 °F)		36.9 °C (98.4 °F)		
TempSrc:	Oral		Oral		
SpO2:	96%	98%	91%	92%
Weight:					
Height:					

Vitals reviewed.
Constitutional:
General: She is not in acute distress.
Appearance: She is well-developed and normal weight. She is not ill-appearing or toxic-appearing.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: Oropharynx is clear. No pharyngeal swelling.

Eyes:
Extraocular Movements: Extraocular movements intact.
Pupils: Pupils are equal, round, and reactive to light.
Neck:
Thyroid: No thyromegaly.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Pulmonary:
Effort: Pulmonary effort is normal. No tachypnea.
Breath sounds: Normal breath sounds.
Chest:
Chest wall: No mass, tenderness, crepitus or edema.
Abdominal:
General: Bowel sounds are normal.
Palpations: Abdomen is soft. There is no mass.
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion and neck supple.
Right lower leg: No tenderness. No edema.
Skin:
General: Skin is warm and dry.
Capillary Refill: Capillary refill takes less than 2 seconds.
Coloration: Skin is not cyanotic.
Findings: No ecchymosis, erythema or rash.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

LABORATORY
[table#4]:
Results from last 7 days
Lab	Units	01/26/24
0536	01/25/24
0531	01/24/24
0548	01/23/24
0544	01/22/24
0531
WBC	K/mcL	5.7	4.6	4.8	4.2	4.8
HGB	g/dL	**8.2***	**7.8***	**8.0***	**7.8***	**8.4***
HEMATOCRIT	%	**26.7***	**25.4***	**26.3***	**25.6***	**26.8***
PLT	K/mcL	**478***	413	364	353	368
NEUTROS PCT AUTO	%	--	--	65.7	57.0	67.6
LYMPHS PCT AUTO	%	--	--	19.3	25.2	17.6
MONOS PCT AUTO	%	--	--	11.1	**14.2***	12.2
EOS PCT AUTO	%	--	--	2.7	2.2	1.2

[table#5]:
Results from last 7 days
Lab	Units	01/26/24
0536	01/25/24
0531	01/24/24
0548
SODIUM	mmol/L	**131***	**132***	**132***
POTASSIUM	mmol/L	4.5	4.3	4.9
CHLORIDE	mmol/L	98	**97***	98
CO2	mmol/L	**32***	**37***	**37***
BUN	mg/dL	16	14	16
CREATININE	mg/dL	0.67	0.70	0.70
CALCIUM	mg/dL	8.4	**8.3***	**8.0***
BILIRUBIN TOTAL	mg/dL	0.3	0.2	0.2
ALK PHOS	Units/L	89	74	74
ALT	Units/L	14	19	17
AST	Units/L	26	27	25
GLUCOSE	mg/dL	100	120	94

MICROBIOLOGY
[table#6]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth Final
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth Final
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Culture	No AFB isolated to Date
	AFB Smear	No AFB seen by fluorescent stain
Fungus culture
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Result	No Fungus Isolated to Date
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth Final
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Culture	No Growth Final
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)
Urine culture
	Collection Time: 01/16/24 2:33 PM

	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
MRSA Surveillance
	Collection Time: 01/19/24 12:30 PM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/25/24 10:34 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth to Date
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen

IMAGING:
MRI cervical spine with and without contrast

Result Date: 1/26/2024
Impression: Contrast imaging is very limited due to extensive motion. Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema. Findings are concerning for discitis/osteomyelitis at C4-C5. release

MRI cervical spine without contrast

Result Date: 1/25/2024
IMPRESSION: 1. Abnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation. 2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels. Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024. release 1419 A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

MRI brain without contrast

Result Date: 1/25/2024
IMPRESSION: 1. No acute intracranial findings. 2. Age-approp
riate mild FLAIR high signal in the white matter likely from microvascular change. 3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process. release

X-ray chest single frontal view

Result Date: 1/25/2024
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated release

X-ray portable chest single frontal view

Result Date: 1/22/2024
IMPRESSION: 1. Right chest tube removal with no pneumothorax. There is stable pleural-parenchymal opacity at the right lung base. 2. Mild left basilar atelectatic versus low-grade inflammatory change. release CC Dr: p: f:

X-ray portable chest single frontal view

Result Date: 1/21/2024
IMPRESSION: Right pleural catheter in place. Small to moderate pleural effusion and patchy right basilar opacity, probably atelectasis. No pneumothorax. release

X-ray portable chest single frontal view STAT

Result Date: 1/20/2024
IMPRESSION: Stable position of right chest tube, some loculated pleural fluid and no pneumothorax. Mild bibasilar volume loss. These findings are concordant with the preliminary interpretation provided by StatRad radiologist, Statrad Code: A release CC Dr: p: f:

X-ray portable chest single frontal view

Result Date: 1/20/2024
IMPRESSION: Right-sided pigtail chest tube remains in place. There is minimal basilar volume loss. Cardiomegaly again noted. Mild redistribution. release CC Dr: p: f:

X-ray foot right 3 or more views

Result Date: 1/19/2024
IMPRESSION: 1. Chronic erosions of the distal first metatarsal likely related to gout 2.. No acute bony pathology. release

ASSESSMENT
[table#7]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension
•	Acute respiratory failure with hypercapnia (CMS/HCC)
•	Acute on chronic systolic heart failure (CMS/HCC)
•	Loculated pleural effusion
•	Atrial fibrillation with rapid ventricular response (CMS/HCC)
•	Empyema of left pleural space (CMS/HCC)
•	Acute osteomyelitis of cervical spine (CMS/HCC)

PLANS/DISCUSSIONS
This is a 59-year-old female with alcohol abuse and chronic nonischemic cardiomyopathy secondary to remote methamphetamine abuse who was admitted to the hospital 14 days ago with sepsis associated with acute respiratory failure. She was found to have severe right lower lobe pneumonia associated with a large pleural effusion consistent with early empyema. The patient also had severe ulcerations in her oral cavity, attributed to herpes infection. The patient received 10 days of intravenous antibiotic treatment for above infection subsequently tapered to Augmentin after significant clinical improvement. Sepsis workups including the pleural fluid culture came back negative. The patient is now having nonspecific headache found to have cervical spine infection. Such infection is likely pre-existing upon admission due to severe oropharyngitis from herpes virus and oral flora. The patient likely had severe aspiration pneumonia again caused by oral and upper GI flora due to repeated nausea vomiting from alcohol abuse. Such polymicrobial infection is likely involved in both pulmonary and current cervical spine infection which progressed from oropharyngeal area. The patient had repeatedly tested negative for MRSA screening making of multidrug-resistant organism involvement unlikely, given no fever or leukocytosis for the past few days. Overall, I suspected that the cervical spine infection is likely caused by the same organism from oral cavity. The patient still has residual pulmonary infection from recent empyema and severe aspiration pneumonia. In light of this, I will recommend the following:

1. Continue high-dose Rocephin for empirical treatment of newly diagnosed C-spine infection a
s well as residual pulmonary infection.
2. Add Flagyl for additional anaerobic coverage of infection by oral flora.
3. Stop vancomycin for no clinical evidence of MRSA infection.
4. ENT evaluation.
5. Trend periodical sedimentation rate and C-reactive protein to assess the progress of empirical antibiotic treatment. The patient likely needs at least 8 weeks of high-dose Rocephin treatment for cervical spine infection.

Thank you for this consultation. I will be happy to follow this patient during the hospital stay.

Dr. Baran will cover the next 2 days on prn basis.

Hai Shao MD PhD

Electronically signed by Hai Shao, MD, PhD at 01/26/2024 4:45 PM PST
Electronically signed by Hai Shao, MD, PhD at 01/26/2024 4:46 PM PST

Howie Que, DPM - 01/20/2024 10:07 PM PST
Associated Order(s): IP CONSULT TO PODIATRY
Formatting of this note might be different from the original.
Subjective

Admission Diagnosis
Oral thrush; Pleural effusion; Prolonged QT interval; COPD exacerbation (C*
HPI

Very pleasant 59-year-old female admitted to the inpatient ward with bilateral lower extremity pain. Patient was at bedside, alert and oriented x 3, in no acute distress. Patient relates an approximately 3-week history of pain localized to the right anterior lateral

ankle. Patient denies any history of trauma or inciting event. Patient states that the present is at all times, including when she is nonweightbearing. Patient rates the pain as severe on the pain scale, and states that it is worse in the evening. Patient has not noticed any open wounds or drainage. Patient has not noticed any local redness and has minimal swelling of the ankle. Patient does not believe patient it is not related to any sort of infection. Patient denies any significant past medical history for diabetes mellitus. Deeply lives at home with family. Condition getting worse over time. Denies calf, thigh, groin pain. Denies chest pain or shortness of breath. Denies constitutional symptoms, no other complaints at this time.

Medical History
Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Surgical History
Elizabeth has no past surgical history on file.

Family History
Elizabeth's family history includes Hypertension in her father.

Social History
Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Medications
• acetaminophen (TYLENOL) tablet 650 mg, 650 mg, oral, Q6H PRN **OR** acetaminophen (TYLENOL) suppository 650 mg, 650 mg, rectal, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• acyclovir (ZOVIRAX) suspension 400 mg, 400 mg, oral, Q8H SCH, Khoo, Jennifer, PharmD, 400 mg at 01/20/24 2051
• benzonatate (TESSALON) capsule 100 mg, 100 mg, oral, TID PRN, Dao, Diep Ngoc, DO, 100 mg at 01/20/24 2051
• Bowel Regimen of PO sennosides-docusate sodium, PR bisacodyl and PO lactulose, , , Until Discontinued **AND** sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet, 1 tablet, oral, BID, 1 tablet at 01/16/24 2145 **AND** bisaCODYL (DULCOLAX) suppository 10 mg, 10 mg, rectal, Daily PRN **AND** lactulose (ENULOSE) 20 gram/30 mL solution 20 g, 20 g, oral, BID PRN, Dammert Coronado, Pedro Manuel, MD
• cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g, 2 g, intraVENOUS, Q24H, Dammert Coronado, Pedro Manuel, MD, 2 g at 01/20/24 2051
• clotrimazole (MYCELEX) troche 10 mg, 10 mg, mucous membrane, TID, Ani, Samuel, MD, 10 mg at 01/20/24 2051
• colchicine (COLCRYS) tablet 0.6 mg, 0.6 mg, oral, Daily, Dao, Diep Ngoc, DO, 0.6 mg at 01/20/24 0845
• dextrose 10 % (D10W) infusion, 40 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and lactated Ringer's (D5LR) infusion, 50 mL/hr, intraVENOUS, Daily PRN, Dammert Coronado, Pedro Manuel, MD
• dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion, 75 mL/hr, intraVENOUS, Continuous PRN, Dammert Coronado, Pedro Manuel, MD
• diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL, 15 mL, mouth/throat, Before meals & at bedtim e, Dammert Coronado, Pedro Manuel, MD, 15 mL at 01/20/24 2051
• [Held by provider] enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg, 40 mg, subcutaneous, Daily, Ani, Samuel, MD, 40 mg at 01/19/24 0824
• furosemide (LASIX) tablet 20 mg, 20 mg, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 20 mg at 01/20/24 1107
• HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg, 0.5 mg, intraVENOUS, Q3H PRN, Dammert Coronado, Pedro Manuel, MD, 0.5 mg at 01/19/24 2039
• ipratropium-albuterol (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL, 3 mL, nebulization, Q4H PRN, Pena, Jose Rafael, MD, 3 mL at 01/20/24 2107
• lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL, 1 spray, mucous membrane, 4x Daily PRN, Umukoro, Emuejevuoke Anastasia, DO, 1 mL at 01/20/24 1348
• lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch, 1 patch, transdermal, Q24H, Umukoro, Emuejevuoke Anastasia, DO, 1 patch at 01/20/24 0525
• melatonin tablet 10 mg, 10 mg, oral, At bedtime PRN, Sassounian, Nicole, DO, 10 mg at 01/16/24 2145
• ondansetron (PF) (ZOFRAN) injection 4 mg, 4 mg, intraVENOUS, Q6H PRN, 4 mg at 01/13/24 0030 **AND** metoCLOPramide (REGLAN) injection 10 mg, 10 mg, intraVENOUS, Q6H PRN **AND** prochlorperazine (COMPAZINE) injection 5 mg, 5 mg, intraVENOUS, Q6H PRN, Dammert Coronado, Pedro Manuel, MD
• metroNIDAZOLE in NaCl (iso-os) (FLAGYL) IVPB 500 mg, 500 mg, intraVENOUS, Q12H, Dammert Coronado, Pedro Manuel, MD, Last Rate: 100 mL/hr at 01/20/24 1100, 500 mg at 01/20/24 1100
• multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet, 1 tablet, oral, Daily, Dammert Coronado, Pedro Manuel, MD, 1 tablet at 01/20/24 0845
• naproxen (NAPROSYN) tablet 500 mg, 500 mg, oral, BID PRN, Ani, Samuel, MD
• oxyCODONE (ROXICODONE) immediate release tablet 5 mg, 5 mg, oral, Q4H PRN, Dammert Coronado, Pedro Manuel, MD, 5 mg at 01/20/24 1659
• zolpidem (AMBIEN) tablet 5 mg, 5 mg, oral, At bedtime PRN, Ani, Samuel, MD, 5 mg at 01/20/24 2051

Allergies
Lactose intolerant [lactose]

Objective

Vital Signs
[table#1]:

Vitals:
	01/21/24 1826
BP:	
Pulse:	103
Resp:	29
Temp:	
SpO2:	

Review of Systems

SH IP POD PHYS EXAM:
Right Foot/Ankle:
Left Foot/Ankle:
Lower extremity focused physical exam:
MSK: Bilateral severe HAV deformity with no pain to palpation or crepitation at the first metatarsal phalangeal joint. Right foot severe hammertoe deformity of the second digit with crossover on the dorsal aspect of the hallux.
VASc: Dorsalis pedis and posterior tibial pulse palpable bilaterally. Capillary refill less than 3 seconds. Digital hair present.
N EUR oh: Gross epicritic sensation intact to distal digits bilaterally.
DER M: Edema +1/4 localized to the right ankle joint. Exquisite tenderness to palpation at the right anterior lateral ankle just lateral to the peroneus tertius and medial to the distal fibula. No surrounding erythema. Not warm compared to contralateral side. No point crepitation/bogginess/fluctuance. No streaking or signs of lymphangitis.

X-ray:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

Assessment/Plan
Very pleasant 59-year-old female with possible right ankle gouty arthropathy.

Plan:
1. Educated patient on clinical course of disease process as well as long-term sequela.
2. Discussed conservative versus surgical treatment modalities.
3. At this time, my clinical suspicion is that patient has underlying gout of the right ankle. Ankle tap performed, and fluid sent to pathology to evaluate for gout crystals. Patient has chronic gouty findings of the first metatarsal phalangeal joint on x-ray including sclerotic margins, rate bites, Cloud sign, and Martell sign, all consistent with gouty arthropathy. I believe that patient may h
ave acute gout attack in the ankle at this time.

[table#2]:
Results from last 7 days
Lab	Units	01/20/24
0535
SODIUM	mmol/L	**129***
POTASSIUM	mmol/L	4.9
CHLORIDE	mmol/L	**96***
CO2	mmol/L	**35***
BUN	mg/dL	12
CREATININE	mg/dL	0.66
GLUCOSE	mg/dL	96
CALCIUM	mg/dL	**8.3***

Problem List
)Principal Problem:
Acute respiratory failure with hypercapnia (CMS/HCC)
Active Problems:
Acute on chronic systolic heart failure (CMS/HCC)
Loculated pleural effusion
Atrial fibrillation with rapid ventricular response (CMS/HCC)

Howie Que, DPM
01/20/2410:07 PM

Electronically signed by Howie Que, DPM at 01/21/2024 7:19 PM PST

Truc Ho, MD - 01/14/2024 11:43 PM PST

Formatting of this note might be different from the original.
[image]

General Medicine History and Physical

PCP: No primary care provider on file.

[table#1]:
CHIEF COMPLAINT

Shortness of Breath (Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB)

[table#2]:
HISTORY OF THE PRESENT ILLNESS

59 y.o.female with hx of HFrEF, asthma, alcohol and meth use disorder presenting with shortness of breath.

She presented 2 days ago with cough, fevers, dyspnea, chest pain. Found to have right sided empyema. Had to be intubated and admitted to ICU. Also had afib RVR requiring dilt gtt. Extubated on 1/14 and now stable for transfer out of ICU.

Patient endorses cough productive of green sputum. Denies shortness of breath or chest pain.

[table#3]:
REVIEW OF SYSTEMS
A 14 point review of systems was completed and is negative other than indicated in HPI

[table#4]:
PAST MEDICAL HISTORY

[table#5]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

[table#6]:
PAST SURGICAL HISTORY

No past surgical history on file.

[table#7]:
ALLERGIES

No Known Allergies

[table#8]:
MEDICATIONS

[table#9]:
**Prior to Admission medications **
Medication	Sig	Start Date	End Date	Taking?	Authorizing Provider
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	**Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310**	**12/23/21**	**1/22/22**		**Arcilla, John-Vincent Lim, MD**
aspirin 81 mg tablet	**Take 1 tablet (81 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310**	**12/24/21**	**1/23/22**		**Arcilla, John-Vincent Lim, MD**
benzonatate (TESSALON) 100 mg capsule	**Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days. Caloptima Mcal ID 93931022D30310**	**12/23/21**	**12/30/21**		**Arcilla, John-Vincent Lim, MD**
carvediloL (COREG) 3.125 mg tablet	**Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day. Caloptima Mcal, ID 93931022D30310**	**12/23/21**	**1/22/22**		**Arcilla, John-Vincent Lim, MD**
furosemide (LASIX) 40 mg tablet	**Take 1 tablet (40 mg total) by mouth daily for 14 days. Caloptima Mcal ID

93931022D30310** | **12/23/21** | **1/6/22** | | **Arcilla, John-Vincent Lim, MD** |
| **lisinopriL (PRINIVIL) 10 mg tablet** | **Take 1 tablet (10 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310** | **12/24/21** | **1/23/22** | | **Arcilla, John-Vincent Lim, MD** |
| **multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet** | **Take 1 tablet by mouth daily. Caloptima Mcal ID 93931022D30310** | **12/23/21** | **1/22/22** | | **Arcilla, John-Vincent Lim, MD** |

[table#10]:
FAMILY HISTORY

[table#11]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

[table#12]:
SOCIAL HISTORY

[table#13]:
| **So
cial History** |
| --- |

[table#14]:
Tobacco Use
•	Smoking status:	Former
•	Smokeless tobacco:	Never
Substance Use Topics
•	Alcohol use:	Not Currently

[table#15]:
Social History

[table#16]:
Substance and Sexual Activity
Drug Use	Not Currently

[table#17]:
Social History

[table#18]:
Social History Narrative
•	Not on file

[table#19]:
PHYSICAL EXAM

Vital Signs:
Temp: [36.7 °C (98.1 °F)-37.1 °C (98.8 °F)] 36.7 °C (98.1 °F)
Heart Rate: [63-89] 63
Resp: [11-42] 15
BP: (89-140)/(66-82) 113/77
FiO2 (%): [35 %-40 %] 36 %
SpO2: 95 %
FiO2 (%): 36 %
O2 Flow Rate (L/min): 4 L/min
Admission Weight: (Pt unable to stand for accurate weight)
BMI (Calculated): 22

Patient Vitals for the past 24 hrs:
[table#20]:
| | BP | Temp | Temp src | Pulse | Resp | SpO2 |

| --- | --- | --- | --- | --- | --- | --- |
01/14/24 2039	—	—	—	63	15	95 %
01/14/24 2000	113/77	36.7 °C (98.1 °F)	Oral	86	17	100 %
01/14/24 1828	108/74	36.8 °C (98.2 °F)	Oral	84	19	99 %
01/14/24 1700	117/81	—	—	81	18	—
01/14/24 1600	102/81	37 °C (98.6 °F)	Axillary	85	18	97 %
01/14/24 1500	97/69	—	—	87	14	98 %
01/14/24 1400	100/73	—	—	88	15	96 %
01/14/24 1300	107/82	—	—	89	22	94 %
01/14/24 1200	109/77	—	—	88	11	95 %
01/14/24 1111	—	—	—	89	**(!) 42**	96 %
01/14/24 1000	100/66	—	—	78	20	97 %
01/14/24 0900	105/75	—	—	74	20	100 %
01/14/24 0800	108/78	37.1 °C (98.8 °F)	Oral	74	20	100 %
01/14/24 0730	96/67	—	—	73	20	100 %
01/14/24 0700	97/69	—	—	72	20	100 %
01/14/24 0630	105/77	—	—	74	20	100 %
01/14/24 0600	140/73	—	—	75	18	100 %
01/14/24 0530	101/72	—	—	74	18	100 %
01/14/24 0500	92/66	—	—	75	20	100 %
01/14/24 0400	100/72	37.1 °C (98.8 °F)	Oral	76	20	99 %
01/14/24 0330	102/76	—	—	73	20	99 %
01/14/24 0300	102/74	—	—	76	20	99 %
01/14/24 0230	101/71	—	—	—	—	—
01/14/24 0200	**(!) 89/68**	—	—	74	20	100 %
01/14/24 0145	91/68	—	—	73	20	100 %
01/14/24 0130	96/70	—	—	72	**(!) 33**	94 %
01/14/24 0115	94/68	—	—	73	20	100 %
01/14/24 0100	102/71	—	—	73	20	100 %
01/14/24 0030	95/76	—	—	72	20	100 %
01/14/24 0015	93/71	—	—	73	23	100 %
01/14/24 0000	92/69	37 °C (98.6 °F)	Oral	75	18	100 %
01/13/24 2345	106/78	—	—	75	**(!) 32**	100 %

Physical Exam:
General-well-developed, well-nourished, tearful
HEENT-EOMI, moist mucous membranes
Neck-no lymphadenopathy
Cardiovascular-regular rate and rhythm
Respiratory- bilateral rhonchi
Gastrointestinal-soft, nondistended, nontender
Musculoskeletal-normal bulk and tone
Skin-warm, dry, acyanotic
Vascular-2+ pulses in all extremities, no pitting edema, normal capillary refill.
Neurologic-no focal deficits
Psychiatric-appropriate mood and affect

[table#21]:
LABORATORY STUDIES

[table#22]:
Blood Gases

[table#23]:
Results from last 7 days
Lab	Units	01/14/24
0347	01/13/24
0712	01/13/24
0501
PH ART		7.45	7.40	**7.10***
PCO2 ART	mm Hg	42	42	**87***
PO2 ART	mm Hg	**56***	**286***	**117***
HCO3 ART	mmol/L	**29.4***	25.8	27.2
BASE EXC ART	mmol/L	**5.5***	1.0	**-2.4***
O2 SAT ART	%	**90.1***	**100.0***	97.0
FIO2		35.0	100.0	60.0

[table#24]:
| **Results fro
m last 7 days** |
| --- |
| Lab | Units | 01/12/24
1944 |
| PH VEN | | **7.26*** |
| PCO2 VEN | mm Hg | **66*** |
| PO2 VEN | mm Hg | 28 |
| HCO3 VEN | mmol/L | **29.5*** |
| BASE EXC VEN | mmol/L | 2.3 |
| O2 SAT VEN | % | **41.8*** |

[table#25]:
Results from last 7 days
Lab	Units	01/13/24
0712
OXYHEMOGLOBIN	%	**98.4***
CARBOXYHEMOGLOBIN	%	**0.4***
METHEMOGLOBIN	%	1.2

[table#26]:
CMP

[table#27]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/14/24
0022	01/13/24
2028	01/13/24
1533	01/13/24
1238	01/13/24
0712
SODIUM	mmol/L	**133***	**134***	**132***	< >	**131***	--
SODIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	**134***
POTASSIUM	mmol/L	4.6	4.6	4.7	< >	**5.8***	--
POTASSIUM, WHOLE BLOOD	mmol/L	--	--	--	--	--	**5.3***
CHLORIDE	mmol/L	104	104	103	< >	102	--
CHLORIDE, WHOLE BLOOD	mmol/L	--	--	--	--	--	98
CO2	mmol/L	26	28	27	< >	**20***	--
BUN	mg/dL	**50***	**53***	**56***	< >	**55***	--
CREATININE	mg/dL	**1.20***	**1.22***	**1.30***	< >	**1.10***	--
ANION GAP	mmol/L	**3***	**2***	**2***	< >	9	--
CALCIUM	mg/dL	**8.1***	**8.3***	**8.0***	< >	**7.9***	--
CALCIUM ION	mmol/L	--	--	--	--	--	1.17
MAGNESIUM	mg/dL	2.3	--	--	--	--	--
PHOSPHORUS	mg/dL	--	2.9	3.5	--	**5.7***	--
GLUCOSE	mg/dL	**144***	**173***	**135***	< >	**347***	--
OSMOLALITY CALC	mOsm/kg H20	292	297	292	< >	301	--
< > = values in this interval not displayed.

[table#28]:
Results from last 7 days
Lab	Units	01/14/24
0022	01/13/24
2028	01/13/24
1238	01/13/24
0514	01/12/24
1939
ALBUMIN	g/dL	**2.6***	**2.7***	**2.7***	< >	3.7
AST	Units/L	--	--	--	--	**70***
ALT	Units/L	--	--	--	--	31
BILIRUBIN TOTAL	mg/dL	--	--	--	--	0.8
ALK PHOS	Units/L	--	--	--	--	**133***
< > = values in this interval not displayed.

[table#29]:
CBC

[table#30]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/13/24
0712	01/13/24
0514	01/12/24
1939
WBC	K/mcL	**14.3***	--	**21.4***	**18.7***
HGB	g/dL	**9.7***	--	**11.8***	13.2
TOTAL HGB	g/dL	--	**11.1***	--	--
PLT	K/mcL	**573***	--	**801***	**799***

[table#31]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/13/24
0514	01/12/24
1939
HEMATOCRIT	%	**30.4***	38.8	43.7
MCV	fL	97	**101***	99
MCHC	g/dL	31.9	**30.4***	**30.2***
RDW	%	14.6	14.9	14.8
MPV	fL	9.6	--	9.2
RBC AUTO	M/mcL	**3.14***	**3.83***	4.40
NRBC AUTO	/100 WBCs	0.0	--	0.0
RBC MORPHOLOGY		--	--	Reviewed
OVALOCYTES		--	--	Few

[table#32]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/12/24
1939
BANDS PCT MAN	%	--	3
NEUTROS PCT AUTO	%	**84.2***	--
NEUTROS PCT MAN	%	--	**93.0***
LYMPHS PCT AUTO	%	**9.7***	--
LYMPHO PCT MAN	%	--	**2.0***
MONOS PCT AUTO	%	**3.0***	--
MONO PCT MAN	%	--	**2.0***
EOS PCT AUTO	%	0.1	--
EOSINO PCT MAN	%	--
0.0
BASOS PCT AUTO	%	0.2	--
BASOS PCT MAN	%	--	0.0

[table#33]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/12/24
1939
PRELIM ABS NEUT CT	K/mcL	**12.01***	**16.76***
NEUTROS ABS	K/mcL	**12.01***	--
SEGS ABS MAN	K/mcL	--	**17.39***
LYMPHS ABS AUTO	K/mcL	1.39	--
LYMPHO ABS MAN	K/mcL	--	**0.37***
MONOS ABS AUTO	K/mcL	0.43	--
MONO ABS MAN	K/mcL	--	0.37
EOS ABS AUTO	K/mcL	0.01	--
EOS ABS MAN	K/mcL	--	0.00
BASOS ABS AUTO	K/mcL	0.03	--
BASOS ABS MAN	K/mcL	--	0.00

[table#34]:
Results from last 7 days
Lab	Units	01/12/24
1939
PLATELET CHECK		Verified by slide scan.

[table#35]:
Coags

[table#36]:
Results from last 7 days
Lab	Units	01/12/24
1939
PROTIME	Seconds	**13.3***
APTT	Seconds	31
INR		1.1

[table#37]:
Renal

[table#38]:
Results from last 7 days
Lab	Units	01/12/24
2103
COLOR U		Yellow
CLARITY U		**Slightly Cloudy***
SPEC GRAV U		1.020
PH U	pH	5.5
GLUCOSE U	mg/dL	Negative
KETONES	mg/dL	Negative
BILIRUBIN U		Negative
BLOOD U		**Moderate***
NITRITE U		**Positive***
LEUKOCYTES U		**Small***
UROBILINOGEN U	mg/dL	0.2
WBC UR HPF	/HPF	**10-20***
RBC UR HPF	/HPF	**6-10***
BACTERIA UR HPF	/HPF	**Few***
SQUAMEPI UR HPF	/LPF	**Few***

[table#39]:
Results from last 7 days
Lab	Units	01/12/24
2103
PROTEIN U	mg/dL	**30***

[table#40]:
Body Fluids

[table#41]:
Results from last 7 days
Lab	Units	01/13/24
0202
SOURCE FL		Pleural cavity, right	Pleural cavity, right	Pleural cavity, right	Pleural cavity, right	Pleural cavity, right
VOLUME FL	mL	17.0
COLOR FL		Light Yellow
APPEARANCE FL		Cloudy

[table#42]:
Results from last 7 days

| Lab | Units | 01/13/24
0202 |
SOURCE FL		Pleural cavity, right	Pleural cavity, right	Pleural cavity, right	Pleural cavity, right	Pleural cavity, right
RBC FLUID	/mcL	20,000
WBC FLUID	/mcL	103,752
NEUTROPHIL FLUID	%	94
LYMPHS FL	%	1
MONO+ MACRO FL	%	5
GLUCOSE FL	mg/dL	<20
PROTEIN FL	g/dL	4.5
ALBUMIN FL	g/dL	2.0
LD FL	Units/L	1,379

[table#43]:
Cardiovascular

[table#44]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/13/24
0514	01/12/24
1939
TROPONIN I	ng/mL	--	0.014	0.029
CK	Units/L	128	**148***	--
NT PRO-BNP	pg/mL	--	--	**13,700***

[table#45]:
Results from last 7 days
Lab	Units	01/13/24
0514
CHOLESTEROL	mg/dL	110
HDL	mg/dL	**33***
LDL CALCULATION	mg/dL	63
TRIGLYCERIDES	mg/dL	72

[table#46]:
Infectious Diseases

[table#47]:
Results from last 7 days
Lab	Units	01/12/24
1939
PROCALCITONIN	ng/mL	**0.29***

[table#48]:
Results from last 7 days
Lab	Units	01/12/24
1939
INFLUENZA A PCR		Not Detected
INFLUENZA B PCR		Not Detected
COVID19 ABBOTT		Not Detected

[table#49]:
Results from last 7 days
Lab	Units	01/13/24
0514
HIV AG/AB COMBO		Non Reactive

[table#50]:
Gastroenterology

[table#51]:

Results from last 7 days
Lab	Units	01/12/24
1939
LIPASE	Units/L	114

[table#52]:
Rheum

[table#53]:
| **
Endocrine** |
| --- |

[table#54]:
Results from last 7 days
Lab	Units	01/14/24
1717	01/14/24
1229	01/14/24
0611	01/13/24
1807	01/13/24
1533
POC GLUCOSE	mg/dL	93	101	94	< >	--
BETA HYDROXYBUTYRATE	mg/dL	--	--	--	--	2.2
< > = values in this interval not displayed.

[table#55]:
Results from last 7 days
Lab	Units	01/12/24
1939
TSH	mcIU/mL	1.260

[table#56]:
Oncology

[table#57]:
Drug levels

[table#58]:
Results from last 7 days
Lab	Units	01/14/24
0502	01/13/24
1533
VANCOMYCIN RM	mcg/mL	16	14

[table#59]:
Other

[table#60]:
MICROBIOLOGY

[table#61]:
Recent Results (from the past 720 hour(s))
Influenza A/B
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value
	Influenza A PCR	Not Detected
	Influenza B PCR	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM

	Specimen: Nasopharyngeal; Swab
Result	Value
	COVID-19, Rapid (PCR/NAAT)	Not Detected
Urine culture
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value
	Urine Culture	
		Multiple organisms present, probable skin/urogenital contamination.
Blood culture
	Collection Time: 01/12/24 9:30 PM
	Specimen: Blood, venous
Result	Value
	Blood Culture	No Growth to Date
Blood culture
	Collection Time: 01/12/24 9:48 PM
	Specimen: Hand, right; Blood
Result	Value
	Blood Culture	No Growth to Date
MRSA Surveillance
	Collection Time: 01/13/24 2:01 AM
	Specimen: Nasal; Swab
Result	Value
	Staph Screen culture	
		NO Methicillin/Oxacillin Resistant Staphylococcus aureus
AFB Culture w/Stain
	Collection Time: 01/13/24 2:01 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	AFB Smear	No AFB seen by fluorescent stain
Wound/Fluid/Tissue Culture with stain
	Collection Time: 01/13/24 2:02 AM
	Specimen: Pleural cavity, right; Body Fluid
Result	Value
	Wound Culture	No Growth to Date
	Gram Stain	Many WBCs
	Gram Stain	No organisms seen
Respiratory culture with stain
	Collection Time: 01/13/24 9:40 PM
	Specimen: Sputum
Result	Value
	Gram Stain	Moderate WBCs
	Gram Stain	No organisms seen
Strep pneumo antigen, urine
	Collection Time: 01/13/24 11:41 PM
	Specimen: Urine, clean catch
Result	Value
	Strep Pneumo Ag,UR	Positive (A)

[table#62]:
IMAGING
X-ray portable chest single frontal view

Result Date: 1/14/2024
IMPRESSION: Essentially unchanged examination. release

[table#63]:
ASSESSMENT AND PLAN

#hypoxic respiratory failure
#sepsis secondary to right sided empyema s/p chest tube 1/13
-extubated 1/14
-pleural fluid was exudative and positive for 103K nucleated cells, bacteria
-pleural fluid cx pending
-strep pneumo ag positive
-continue chest tube to suction
-pulm following
-continue rocephin, flagyl, vanc

-SLP given concerns for aspiration

#afib RVR
No documented hx of afib that I can see
-s/p amio gtt
-consider DOAC although CHADVASC only 2

#HFrEF, not in exacerbation
Echo shows severe LV dilation, EF 25%. Also, RV EF mildly decreased.
-spot dose lasix daily as needed to keep net negative.
-hold coreg and lisinopril given soft BP.

#AKI
Cr 1.2 from baseline of 0.9. suspect related to sepsis. Cardiorenal is possible as well
-treat as above

#asthma
-duone
bs as needed

#mouth sores
-HIV negative
-continue BLM mouth wash

Code Status: Full Code

[table#64]:
Emergency Contacts

[table#65]:
	Contact Person (Rel.)	Home Phone	Work Phone	Mobile Phone
	DIMARELLO,ERIKA (Daughter)	--	--	609-802-3373
	Dimarello,Anthony (Son)	--	--	949-556-1803

[table#66]:

FEN: NPO Diet NPO; Ice Chips, Sips With Meds, Sips Of Clear Liquids

Quality Checklist:
Telemetry: [ ]None [x]Present/Indication:
Foley: [x]None [ ]Present/Indication:
CVC/PICC: [x]None [ ]Present/Indication:
DVT Prophylaxis: [ ]SCDs [ ]Aspirin [ ]LMWH [x]SQ Heparin [ ]Full anticoagulation [ ]Contraindicated [ ]Ambulate TID
Plan of care discussed with: [ ]Consultant [ ]PCP [ ]DPOA [ ]Family
Risk of morbidity/mortality: [ ]Low [x]Moderate [ ]High

Truc Viet Ho, MD

Electronically signed by Truc Ho, MD at 01/15/2024 12:21 AM PST

Associated Order(s): IP CONSULT TO VASCULAR ACCESS
Summary: RIGHT BASILIC VEIN MIDLINE INSERTION

Formatting of this note might be different from the original.
Powerglide Midline Catheter Placement Notes

Order by Dr. Christopher Jones was verified in chart and with primary nurse Jack Verkuilen, RN.

Patient was intubated and sedated. Right upper arm examined, brachial complex visualized and patient position to optimize desired vein's view. Right basilic vein accessed. Sterile technique observed.
Powerglide Midline Catheter 18G placed using ultrasound guided technique on sterile field. Catheter advanced at 10cm mark. Adequate blood flow noticed after insertion. Device secured, biopatch applied and large Tegaderm used as dressing. Patient remained stable during and after the procedure. Report given to patient's primary nurse.

Complication: None.

Glenn Joseph Interno, RN, BSN, CCRN-CMC
Department of Critical Care

Electronically signed by Your Scripps Health Team at 01/14/2024 2:35 AM PST

Nursing Notes - documented in this encounter

Your Scripps Health Team - 02/17/2024 4:00 PM PST
Formatting of this note might be different from the original.
Patient discharged to Mission Hills Post acute care,room 27B. Called report to nurse Maria (6195770917) and went over pertinent information, answered questions. Telebox removed. Patient left floor wnl vital signs,in no apparent distress via wheelchair accompanied by EMT's with all paperwork.

Electronically signed by Your Scripps Health Team at 02/17/2024 4:04 PM PST

ED Notes - documented in this encounter

Stephen Zanoni, MD - 01/12/2024 7:03 PM PST
Formatting of this note might be different from the original.

Emergency Department Record
SCRIPPS MERCY HOSPITAL, CHULA VISTA

Date of Service: 1/12/2024
**Name: **Elizabeth Tina Petruccio
**MRN: **810760324
CSN: 10143864623
**DOB: **7/29/1964 **Age: **59 y.o.

The patient arrived by ambulance and is alone.

[table#1]:
Chief Complaint
Patient presents with
•	Shortness of Breath
		Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB
Chief complaint is respiratory distress

History of Present Illness:
Elizabeth Tina Petruccio is a 59 y.o. female who comes to us via ambulance due to shortness of breath
This patient usually lives in Long Beach with her son but she was in Mexico
Over the last week she has been feeling unwell with shortness of breath cough and chest pain and chest pressure
She reports fevers as well

She went to hospital in Mexico she did not like the care the treatment she was given so she came to the night states called ambulance
On ambulance they did not give albuterol as they did not hear wheezes but did give oxygen for comfort at 4 L

She states that she has a history of CHF, asthma, and atrial fibrillation

Review of Systems:

All other systems were reviewed and negative except for those mentioned in the history of present illness.

[table#2]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Medications:
[table#3]:
Current Outpatient Medications

[table#4]:
	albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler
	aspirin 81 mg tablet
	benzonatate (TESSALON) 100 mg capsule
	carvediloL (COREG) 3.125 mg tablet
	furosemide (LASIX) 40 mg tablet
	lisinopriL (PRINIVIL) 10 mg tablet
	multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet

[table#5]:

Allergies:
No Known Allergies

Social History:
Elizabeth Tina Petruccio reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

Surgical History:
No past surgical history on file.

Family History:
[table#6]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

Physical Exam:
[table#7]:
Vitals:
	01/12/24 2001	01/12/24 2002	01/12/24 2023	01/12/24 2134
BP:				111/90
Pulse:	124			128
Resp:	28			24
Temp:				36.6 °C (97.8 °F)
TempSrc:				Oral
SpO2:	99%	99%	98%	100%
Weight:				
Height:				

General: In distress, increased work of breathing
HENT: Normocephalic and atraumatic; mucous membranes are moist, thrush is present
Back: No CVA tenderness
Eyes: Extraocular muscles are intact, pupils are equal, round, and reactive.
Neck: Supple, full range of motion
Cardiovascular: Tachycardia, irregular
Pulmonary: When the patient is breathing and expiratory stridor is heard, on lung exam the expiratory phase is prolonged, but she has poor air movement
Abdominal: Soft, non-tender, non-distended; no masses; no rebound or guarding
Extremities: Warm, well perfused, no edema

Skin: No rashes or bruising
Neurological: GCS 15, A and O x4, no dysarthria facial expression normal patient moves all extremities spontaneously
Vascular: Normal cap refill good pulses

**Labs: **
[table#8]:
Recent Results (from the past 24 hour(s))
12-Lead ECG
	Collection Time: 01/12/24 7:36 PM
Result	Value	Ref Range
	VENTRICULAR RATE EKG/MIN	152
bpm
	QRS-INTERVAL (MSEC)	98	ms
	QT-INTERVAL (MSEC)	318	ms
	QTC	505	ms
	R AXIS	69	degrees
	T AXIS	212	degrees
Comprehensive metabolic panel
	Collection Time: 01/12/24 7:39 PM
Result	Value	Ref Range
	Fasting?		
	Glucose	132 (H)	70 - 125 mg/dL
	Sodium	137	137 - 145 mmol/L
	Potassium	5.1	3.5 - 5.1 mmol/L
	Chloride	100	98 - 107 mmol/L
	CO2, Total	32 (H)	22 - 30 mmol/L
	Urea Nitrogen	48 (H)	7 - 17 mg/dL
	Creatinine	1.09 (H)	0.50 - 1.00 mg/dL
	CKD-EPI eGFR 2021	59 (L)	>60 mL/min/1.73m*2
	Anion Gap	5 (L)	6 - 14 mmol/L
	Osmolality Calculated	298	280 - 305 mOsm/kg H20
	Calcium	9.6	8.4 - 10.2 mg/dL
	Protein, total	8.8 (H)	6.3 - 8.2 g/dL
	Albumin	3.7	3.5 - 5.0 g/dL
	Alkaline Phosphatase	133 (H)	38 - 126 Units/L
	AST	70 (H)	14 - 36 Units/L
	ALT	31	<=34 Units/L
	Bilirubin, Total	0.8	0.2 - 1.3 mg/dL
Prothrombin time-INR
	Collection Time: 01/12/24 7:39 PM
Result	Value	Ref Range
	Protime	13.3 (H)	10.0 - 13.1 Seconds
	INR	1.1	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation
Activated PTT
	Collection Time: 01/12/24 7:39 PM
Result	Value	Ref Range
	aPTT	31	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds
Troponin I
	Collection Time: 01/12/24 7:39 PM
Result	Value	Ref Range
	Troponin I	0.029	<=0.034 ng/mL
NT Pro-BNP
	Collection Time: 01/12/24 7:39 PM
Result	Value	Ref Range
	NT pro-BNP	13,700 (H)	Refer to the reference range comment. pg/mL
CBC with Differential
	Collection Time: 01/12/24 7:39 PM
Result	Value	Ref Range
	WBC	18.7 (H)	3.4 - 11.0 K/mcL
	RBC	4.40	3.98 - 5.25 M/mcL
	HGB	13.2	11.9 - 15.3 g/dL
	HCT	43.7	37.3 - 46.7 %
	MCV	99	81 - 100 fL
	MCH	30	26 - 33 pg
	MCHC	30.2 (L)	31.0 - 36.0 g/dL
	RDW	14.8	11.7 - 14.9 %
	Platelet Count	799 (H)	150 - 425 K/mcL
	MPV	9.2	9.0 - 12.8 fL
	nRBC	0.0	0.0 /100 WBCs

| | Preliminary Abs Neut | 16.76 (H) | 1.50 - 7.40 K/mcL |
| **Influenza A/B** |
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	Influenza A PCR	Not Detected	Not Detected
	Influenza B PCR	Not Detected	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 01/12/24 7:39 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected
TSH (ED Mercy only)
	Collection Time: 01/12/24 7:39 PM
Result	Value	Ref Range
	Thyroid Stimulating Hormone	1.260	0.465 - 4.680 mcIU/mL
Manual Differential,Reflexed
	Collection Time: 01/12/24 7:39 PM
Result	Value	Ref Range
	Neutrophils %	93.0 (H)	38.0 - 74.0 %
	Bands %	3	0 - 6 %
	Lymphocytes %	2.0 (L)	16.0 - 48.0 %
	Monocytes %	2.0 (L)	4.9 - 12.5 %
	Eosinophils %	0.0	0.0 - 9.5 %
	Basophils %	0.0	0.0 - 1.6 %
	Absolute Neutrophils	17.39 (H)	1.50 - 7.40 K/mcL
	Absolute Bands	0.56	<=0.60 K/mcL
	Absolute Lymphocytes	0.37 (L)	0.90 - 3.10 K/mcL
	Absolute Monocytes	0.37	0.26 - 0.87 K/mcL
	Absolute Eosinophils	0.00	0.00 - 0.51 K/mcL
	Absolute Basophils	0.00	0.00 - 0.09 K/mcL
	PLT Slide Review	Verified by slide scan.	
	RBC Slide Review	Reviewed	
	Ovalocytes	Few	
**Blood ga
s lactic acid, venous**
	Collection Time: 01/12/24 7:44 PM
Result	Value	Ref Range
	pH, Venous	7.26 (CL)	7.35 - 7.45
	pCO2, Venous	66 (CH)	38 - 50 mm Hg
	pO2, Venous	28	Not Established mm Hg
	HCO3, Venous	29.5 (H)	22.0 - 29.0 mmol/L
	O2 Sat, Venous	41.8 (L)	60.0 - 85.0 %
	Base Excess/Deficit, Venous	2.3	Not Established mmol/L
	Lactic Acid, Venous	2.9 (H)	0.9 - 2.5 mmol/L
	Notification Note	CRITICAL VPH/VPCO2	
	Notified By	128296	
	Notified Whom	EMMA 241391 ER RN	
	Operator ID	128,296	
Urinalysis, culture if indicated
	Collection Time: 01/12/24 9:03 PM
	Specimen: Urine, clean catch
Result	Value	Ref Range
	Color (UA)	Yellow	Yellow
	Clarity (UA)	Slightly Cloudy (A)	Clear
	Specific Gravity (UA)	1.020	1.005 - 1.030
	pH (UA)	5.5	5.0 - 8.5 pH
	Protein (UA)	30 (A)	Negative mg/dL
	Glucose (UA)	Negative	Negative mg/dL
	Ketones (UA)	Negative	Negative mg/dL
	Bilirubin (UA)	Negative	Negative
	Occult Blood (UA)	Moderate (A)	Negative
	Leukocyte Esterase (UA)	Small (A)	Negative
	Nitrite (UA)	Positive (A)	Negative
	Urobilinogen (UA)	0.2	<2.0 mg/dL
Drug Screen,Urine
	Collection Time: 01/12/24 9:03 PM
Result	Value	Ref Range
	Benzodiazepines (UR)	Negative	Negative ng/mL
	Cocaine (UR)	Negative	Negative ng/mL

	Tetrahydrocannabinol (UR)	Negative	Negative ng/mL
	Opiates (UR)	Negative	Negative ng/mL
	Methamphetamine (UR)	Negative	Negative ng/mL
	Barbiturates (UR)	Negative	Negative ng/mL
	Amphetamine (UR)	Negative	Negative ng/mL
	Methadone (UR)	Negative	Negative ng/mL
	Phencyclidine (UR)	Negative	Negative ng/mL
	Tricyclic Antidepressants (UR)	Negative	Negative ng/mL
	Oxycodone (UR)	Negative	Negative ng/mL
Fentanyl, Urine
	Collection Time: 01/12/24 9:03 PM
Result	Value	Ref Range
	Fentanyl, Interpretation (Ur)	Negative	Negative
Urinalysis Microscopic W/ Reflex to Culture
	Collection Time: 01/12/24 9:03 PM
Result	Value	Ref Range
	WBC (UA)	10-20 (A)	0 - 2 /HPF
	RBC (UA)	6-10 (A)	0 - 2 /HPF
	Epithelial Cells, Squamous (UA)	Few (A)	None /LPF
	Bacteria (UA)	Few (A)	None /HPF

Imaging:
[table#9]:
X-ray portable chest single frontal view
Final Result
IMPRESSION:
Large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly.
release

On the right she has a very large pleural effusion, with what appears to be a pneumonia or airspace disease just just into it, no rib fracture no pneumothorax this is a chest x-ray interpreted by myself 1 view

EKG: EKG interpreted by myself shows atrial fibrillation with rapid ventricular response LVH rate is 152 QRS is 98 QTc is 505, septal and infarct age undetermined with Q waves in the septal leads overall impression abnormal EKG

ED Course And Medical Decision Making:
59-year-old female comes in from out of town with a history of asthma also heart failure and in respiratory distress
Presentation appears to be more with asthma and COPD
Albuterol Atrovent has been started along with magnesium and BiPAP
Patient was also started on diltiazem due to the A-fib with RVR that she was having as well
Patient appears to be having a large pleural effusion, with a pneumonia
Antibiotics have been started
Patient improved but we will still keep her on BiPAP likely admission to either stepdown or ICU
**Also started on diltiazem drip f
or her A-fib with RVR**

QTc also respected magnesium sulfate given IV

Critical Care Time:
80-85 minutes of critical care time was spent on this patient in excess of other billable procedures. This included time documenting the case, review of records, discussion of care with admitting providers. It included multiple reassessments at the bedside and evaluation of patient in response to aforementioned therapies. This case represents a high level of medical decision making and the diagnosis caries a significant morbidity and mortality.

Clinical Impressions:
[table#10]:
1.	**Acute respiratory failure with hypercapnia (CMS/HCC) **
2.	COPD exacerbation (CMS/HCC)
3.	Pleural effusion
4.	Oral thrush
5.	Acute kidney injury (CMS/HCC)
6.	Prolonged QT interval

**Disposition:

Admit**

Zanoni, Stephen Michael, MD
01/12/24 2230

Electronically signed by Stephen Zanoni, MD at 01/12/2024 10:30 PM PST

Kaitlin Pala, MD - 01/12/2024 7:03 PM PST

Formatting of this note might be different from the original.

I discussed this patient's case with ICU Dr. Jones regarding large right pleural effusion and need for thoracentesis. Patient is unable to provide consent and her family members are not currently answering to provide ancillary consent. I did not evaluate this patient independently, but I reviewed her vital signs, chest x-ray and blood work and agree with need for emergent thoracentesis.

Pala, Kaitlin Dveira, MD
01/12/24 2352

Electronically signed by Kaitlin Pala, MD at 01/12/2024 11:52 PM PST

Miscellaneous Notes - documented in this encounter

Plan of Care - Your Scripps Health Team - 02/17/2024 7:40 AM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: Patient remain alert and oriented x4, able to communicate needs. Breathing even and non labored. Denies any pain/discomfort at this time. Vital signs wnl. Patient in no apparent distress at this time. Call light and personal belongings within easy reach.

Key findings related to barriers to transfer/discharge: placement

Patient/Family needs and concerns: none

Electronically signed by Your Scripps Health Team at 02/17/2024 7:40 AM PST

Plan of Care - Your Scripps Health Team - 02/17/2024 12:27 AM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: A/O x 4; denies chest pains or SOB; floor pt; on fluid restriction 2L/day; per pt info, excited to be transferred to SNF in the morning; encouraged to turn q 2 hrs for comfort; fall precautions observed.

Key findings related to barriers to transfer/discharge: Medical clearance

Patient/Family needs and concerns: None

Electronically signed by Your Scripps Health Team at 02/17/2024 12:29 AM PST

Plan of Care - Your Scripps Health Team - 02/16/2024 10:00 AM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: A/OX4, resting comfortably in chair. VSS. RA. Denies chest pain or SOB but pt reported pain on hip and shoulder. PRN pain med given with effect. Edema +3 on BUE and on diuresis. No distress at this time. All safety precautions in place.

**Key findings related to barriers to transfer/discharge: **Placement

Patient/Family needs and concerns: All needs met

Electronically signed by Your Scripps Health Team at 02/16/2024 10:02 AM PST

Plan of Care - Your Scripps Health Team - 02/16/2024 4:35 AM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: A&O x4,Pleasant, follows commands. VSS, O2-RA-2L at night, Non tele. Pt denies chest pain, SOB, dizziness, N/V, diarrhea. Safety measures in place, will continue with POC.

**Key findings related to barriers to transfer/discharge: **Placement

Patient/Family needs and concerns: None at this time

Electronically signed by Your Scripps Health Team at 02/16/2024 4:35 AM PST

Plan of Care - Your Scripps Health Team - 02/15/2024 7:47 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Patient remain alert and oriented x4, able to communicate needs. Breathing even and non labored. Denies any pain/discomfort at this time. Vital signs wnl. Patient in no apparent distress at this time. Call light and personal belongings within easy reach.

**Key findings related to barriers to transfer/discharge: pending placement. **

Patient/Family needs and concerns: none

Electronically signed by Your Scripps Health Team at 02/15/2024 7:47 AM PST

Plan of Care - Your Scripps Health Team - 02/15/2024 12:00 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events:

Pt AAOX4, on RA, no complaints of SOB. Nontele. With complaints of pain to back and L shoulder well managed with pain medication as per MAR. PRN sleeping aid administered, effective. No acute events overnight. Adhering to FR. No acute events at this time. Fall and safety precautions in place, call light within reach, educated pt on use of call light for assistance.

**Key findings related to barriers to transfer/discharge: **

Placement, discharge planning.

Patient/Family needs and concerns:

None identified at this time.

Electronically signed by Your Scripps Health Team at 02/15/2024 12:06 AM PST

Plan of Care - Your Scripps Health Team - 02/14/2024 8:15 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Patient remain alert and oriented x4, able to communicate needs. Breathing even and non labored. Denies any pain/discomfort at this time. Vital signs wnl. Patient in no apparent distress at this time. Call light and personal belongings within easy reach.

Key findings related to barriers to transfer/discharge: medical clearance

Patient/Family needs and concerns: none

Electronically signed by Your Scripps Health Team at 02/14/2024 8:15 AM PST

Plan of Care - Nurse Prescilla S - 02/13/2024 11:52 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: AOX4,no distress noted,stayed mostly in chair,bilateral feet,red and swollen,vital signs stable.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: none

Electronically signed by Nurse Prescilla S at 02/13/2024 11:53 PM PST
Electronically signed by Nurse Prescilla S at 02/14/2024 4:38 AM PST

Plan of Care - Your Scripps Health Team - 02/13/2024 2:02 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt lying in bed, allowed to assume comfortab
le position. With even and unlabored breathing. Does not appear in distress. Given PRN oxycodone-acetaminophen for complaints of pain and effective. No acute changes reported at this time.

**Key findings related to barriers to transfer/discharge: **pending placement

Patient/Family needs and concerns: No concerns reported at this time.

Electronically signed by Your Scripps Health Team at 02/13/2024 2:10 PM PST

Plan of Care - Nurse Prescilla S - 02/13/2024 12:24 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: AOX4,no distress noted,medicated x2 for neck and shoulder pain with relief,very conversant,stable.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: none

Electronically signed by Nurse Prescilla S at 02/13/2024 12:25 AM PST
Electronically signed by Nurse Prescilla S at 02/13/2024 3:31 AM PST

Plan of Care - Your Scripps Health Team - 02/12/2024 12:35 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt sitting on chair, allowed to assume comfortable position. With even and unlabored breathing. Does not appear in distress. Given PRN oxycodone-acetaminophen for complaints of neck pain, and helpful. No acute changes reported at this time.

**Key findings related to barriers to transfer/discharge: **pending placement

Patient/Family needs and concerns: No concerns reported at this time.

Electronically signed by Your Scripps Health Team at 02/12/2024 12:38 PM PST

Plan of Care - Your Scripps Health Team - 02/11/2024 8:15 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt a&ox4, verbally responsive and able to communicate needs well. Breathing even and non labored on RA, no sob or distress noted. Denies any pain at this time. Oriented to room and use of call light, verbalized understanding. Safety measures in place, call light within reach. Needs met and will continue to monitor.

**Key findings related to barriers to transfer/discharge: **Medical clearance

Patient/Family needs and concerns: None

Electronically signed by Your Scripps Health Team at 02/11/2024 11:27 PM PST

Plan of Care - Your Scripps Health Team - 02/11/2024 3:20 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt lying in bed, with even and unlabored breathing. Does not appear in distress. Given PRN oxycodone-acetaminophen for complaints of pain on neck. Pt verbalizes this med is "very helpful with the pain compared to the other medications before." No acute changes noted at this time. Encouraged to call for assist if needed.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: No concerns reported at this time.

Electronically signed by Your Scripps Health Team at 02/11/2024 3:22 PM PST

Plan of Care - Your Scripps Health Team - 02/10/2024 8:30 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt a&ox4, verbally responsive and able to communicate needs well. Breathing even and non labored on RA, no sob or distress noted. Denies any pain at this time. Oriented to room and use of call light, verbalized understanding. Safety measures in place, call light within reach. Needs met and will continue to monitor.

Key findings related to barriers to transfer/discharge: Medical clearance

Patient/Family needs and concerns: None

Electronically signed by Your Scripps Health Team at 02/11/2024 1:36 AM PST

Plan of Care - Your Scripps Health Team - 02/10/2024 12:13 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt A&Ox4. VSS. ST on tele; denies chest pain/SOB. On RA. +3 edema on RLE and +2 on LLE. Lasix administered as ordered; great output noted this AM. RUA PICC line in place for long term ABX. X1 norco administered d/t neck/back. All safety measures in place. Will continue plan of care.

**Key findings related to barriers to transfer/discharge: **Placement

Patient/Family needs and concerns: Updates prn

Electronically signed by Your Scripps Health Team at 02/10/2024 12:16 PM PST

Plan of Care - Your Scripps Health Team - 02/10/2024 1:44 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt AOx4, cooperative with plan of care, denied pain, denied sob. Educated pt to elevate BLE for comfort and to reduce swelling. All needs attended. Ensured call button within reach. Ensured call button within reach
@0229 Medicated with prn pain meds for generalized pain, with relief noted.
Key findings related to barriers to transfer/discharge: placement

Patient/Family needs and concerns: none at this time

Electronically signed by Your Scripps Health Team at 02/10/2024 1:49 AM PST
Electronically signed by Your Scripps Health Team at 02/10/2024 2:45 AM PST
Electronically signed by Your Scripps Health Team at 02/10/2024 3:50 AM PST

Plan of Care - Your Scripps Health Team - 02/09/2024 11:47 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt A&Ox4. VSS. On RA. SR on tele +3 edema on BLE - cleansed bilateral feel this AM. 2L FR. x1 Norco administered for back/neck pain w/ relief prior to physical therapy. RUA PICC line flushed. All safety measures in place. Will continue plan of care.

@1610: Pt had 7 beats of v-tach. MD aware. VS stable - see flowsheets for more. No new orders placed. Pt was crying upon arrival to room, otherwise stable.

Key findings related to barriers to transfer/discharge: Placement

Patient/Family needs and concerns: Updates prn

Electronically signed by Your Scripps Health Team at 02/09/2024 11:56 AM PST
Electronically signed by Your Scripps Health Team at 02/09/2024 6:57 PM PST

Plan of Care - Your Scripps Health Team - 02/09/2024 2:45 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress
** **
Important trends/observation and significant events:
Pt A&Ox4. VSS - 93% on RA, NSR on tele. No apparent signs of distress. Pt denies chest pain, SOB, palpitations. Reports intermittent pain in RU shoulder relieved with prn Norco and rest. Pt requested ambien for sleep. Decreased sensation in both feet. Fall precautions in place, medication given, education provided.
** **
**Key findings related to barriers to transfer/discharge: **
Medical clearance, SNF placement

Patient/Family needs and concerns:
None at this time.

Cosigned by Your Scripps Health Team at 02/09/2024 5:40 AM PST
Electronically signed by Your Scripps Health Team at 02/09/2024 3:03 AM PST
Electronically signed by Your Scripps Health Team at 02/09/2024 5:40 AM PST

Plan of Care - Your Scripps Health Team - 02/08/2024 7:57 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: A/O x4, NSR on monitor, no SOB on room air.
Continue with POC: Fluid restriction, diurese, IV and oral antibiotics.
BRP with steady gait, call light within reach, medicated with Norco for pain with good releif. Will monitor.

**Key findings related to barriers to transfer/discharge: **Medical clearance, SNF placement

Patient/Family needs and concerns:

Electronically signed by Your Scripps Health Team at 02/08/2024 5:40 PM PST

Plan of Care - Your Scripps Health Team - 02/08/2024 1:27 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress
** **
Important trends/observation and significant events:
Pt A&Ox4. VSS - 94% while awake, requires 1L NC when asleep to maintain SPO2. NSR/ST on tele. No apparent signs of distress. Pt denies chest pain, SOB, palpitations. Reports intermittent pain in RU shoulder relieved with rest and heat packs. Decreased to no sensation in both feet. Pt requested ambien for sleep. Fall precautions in place, medication given, education provided.
** **
**Key findings related to barriers to transfer/discharge: **
Medical clearance, SNF placement

Patient/Family needs and concerns:
None at this time.

Cosigned by Your Scripps Health Team at 02/08/2024 6:16 AM PST
Electronically signed by Your Scripps Health Team at 02/08/2024 1:36 AM PST
Electronically signed by Your Scripps Health Team at 02/08/2024 6:16 AM PST

Plan of Care - Nurse Feliccia T - 02/07/2024 12:19 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events:
Pt A&Ox4, Satting 100% on RA, ST/SR on Tele, mo signs of acute distress. Contacted RT to perform breathing Tx for pt.
PRN Dilaudid give for neck pain, pt stated that her neck pain was still present. RN received new orders and PRN norco was given to pt with effective results.
Safety measures in place; bed on lowest position, call light within reach.

**Key findings related to barriers to transfer/discharge: **
Medical clearannce, SNF placement

Patient/Family needs and concerns:
None at this time.

Electronically signed by Nurse Feliccia T at 02/07/2024 12:23 PM PST

Plan of Care - Your Scripps Health Team - 02/07/2024 4:00 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt A/O x4, calm and cooperative. c/o pain on mid neck, medicated with prn pain medication. On RA, no respiratory distress, no SOB. Afebrile. On tele NSR/ST ,no cardiac symptoms, denies chest pain. Safety and fall precautions in place, call light within easy reach.

**Key findings related to barriers to transfer/discharge: **Medical clearance

Patient/Family needs and concerns: None at this time

Electronically signed by Your Scripps Health Team at 02/07/2024 4:01 AM PST

Plan of Care - Your Scripps Health Team - 02/06/2024 12:51 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt A&Ox4, calm and cooperative. c/o pain on mid neck, medicated with prn pain medication as ordered. 98% O2sat on RA, no respiratory distress, no SOB. Afebrile. On tele with QTC 433 NSR ST,no cardiac symptoms, denies chest pain. Safety and fall precautions in place, call light within easy reach. Will continue plan of care.

OT will b
e d'c, goals are met and pt is independent per OT,

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: None at this time

Electronically signed by Your Scripps Health Team at 02/06/2024 12:57 PM PST
Electronically signed by Your Scripps Health Team at 02/06/2024 1:02 PM PST

Plan of Care - Your Scripps Health Team - 02/06/2024 12:06 AM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt is alert and oriented X 4. SR on tele monitor. PRN pain meds and non-pharmaceutical measures to help control pain.PRN med for insomnia.

**Key findings related to barriers to transfer/discharge: **Medical clearance, Placement

Patient/Family needs and concerns: Pt reports of pain on her neck radiating to her right shoulder and right upper arm. Per pt, has this pain for several days. Requested to notify MD. MD notified.

Electronically signed by Your Scripps Health Team at 02/06/2024 12:21 AM PST

Plan of Care - Your Scripps Health Team - 02/05/2024 5:32 PM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: A&Ox4, HR low 100's. C/o pain in neck & left hip, PRN pain meds given x1. IV abx continued. Tolerating diet well, ambulating to bathroom independently, voiding & 1 2 BM's this shift.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: pt reporting 1 year history of upper abdominal pain occurring intermittently, occurring a few hours after eating. Has concerns if it is related to her gallbladder and is reporting the pain episodes have occurred 3 times during this admission. Pt reported this to MD in AM.

Electronically signed by Your Scripps Health Team at 02/05/2024 5:38 PM PST

Plan of Care - Your Scripps Health Team - 02/05/2024 12:04 AM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt is alert and oriented X 4. SR/St on tele monitor. PRN med for pain and anxiety. On 2000 ml/day Fluid restriction. Denies chest pain.

**Key findings related to barriers to transfer/discharge: **Medical clearance

Patient/Family needs and concerns:

Electronically signed by Your Scripps Health Team at 02/05/2024 12:12 AM PST

Plan of Care - Your Scripps Health Team - 02/04/2024 11:24 AM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt A&Ox4, calm and cooperative with care. VSS. 95% O2sat RA, no respiratory distress, no SOB. On tele, QTC yellow @491 Afib w/ PVC, no cardiac symptoms, denies chest pain. Afebrile. Pt requested diet change from dental soft diet to regular diet, pt claims she is able to tolerate regular food texture. On FR 2000mL. Ambulated on the hallway; tolerated well. Fall and safety precautions in place. Will continue plan of care

0835 pt c/o neck pain 8/10, given pain medication as ordered with some relief. Pt claims 2/10 pain relief.

**Key findings related to barriers to transfer/discharge: ** Medical clearance

Patient/Family needs and concerns: None at this time.

Electronically signed by Your Scripps Health Team at 02/04/2024 11:35 AM PST
Electronically signed by Your Scripps Health Team at 02/04/2024 4:41 PM PST
Electronically signed by Your Scripps Health Team at 02/04/2024 5:12 PM PST

Plan of Care - Your Scripps Health Team - 02/04/2024 6:48 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: A&Ox4, VSS on room air. Denies pain and acute distress, no s/s of acute distress. PRN medications for pain. FR 1.5L. PRN breathing tx. Fall and safety precautions in place. Will continue to monitor.

**Key findings related to barriers to transfer/discharge: **Medical clearance

Patient/Family needs and concerns:

Electronically signed by Your Scripps Health Team at 02/04/2024 6:49 AM PST

Plan of Care - Your Scripps Health Team - 02/03/2024 3:41 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt A&Ox4. NSR on tele, VSS, afebrile. No s/s of acute distress. Continuing w/ POC. FR 1.5L. PRN breathing tx. CHG bath completed. IV abx administered as ordered. Fall precautions in place. Will continue to monitor.

**Key findings related to barriers to transfer/discharge: **med clearance

Patient/Family needs and concerns: all needs met

Electronically signed by Your Scripps Health Team at 02/03/2024 3:49 PM PST

Plan of Care - Your Scripps Health Team - 02/02/2024 11:13 PM PST

Formatting of this note might be different from the original.
Patient is continuing with the plan of care. Pt VSS, telemetry in place, RA, A&Ox4. Patient receiving ATBX therapy. PRNs included Zofran at 2106 for nausea, oxycodone at 2122 for back pain level of 4/10 (re-assessed pain of 0/10, pain management effective), calcium carbonate at 2251 for heartburn, melatonin at 0200 for sleep aid. Pt placed on NC @ 2L for desatting during sleep, O2sat subsequently increased to >94%. Pt's needs are met at this time, call light w
ithin reach, bed locked and in the lowest position. Focus on pain management, ATBX tx, placement pending. Black watery stool at 0119, will endorse to day shift.

Cosigned by Nurse Esther N at 02/09/2024 3:46 AM PST
Electronically signed by Your Scripps Health Team at 02/03/2024 2:42 AM PST
Electronically signed by Nurse Esther N at 02/03/2024 3:38 AM PST
Electronically signed by Your Scripps Health Team at 02/03/2024 4:20 AM PST
Electronically signed by Nurse Esther N at 02/09/2024 3:46 AM PST

OT Treatment Note - Elaheh S - 02/02/2024 3:07 PM PST

Formatting of this note might be different from the original.
** Occupational Therapy**
[image]

Patient Name: Elizabeth Tina Petruccio
Admission Date: 1/12/2024
Treatment Date: 2/2/2024
Code Status: Full Code
Unit/Room: 0426/01

**Diagnosis: **The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

Social History
Type of Residence: Shelter
Assistive Device : (none)
Living Arrangements: Alone
Communication: WNL

Prior Level of Function
Functional Mobility: Independent
ADLs: Independent

[table#1]:
Subjective / Objective / Assessment

[table#2]:
	02/02/24 1507
OT Treatment
OT Treatment Start Time	1452
OT Treatment End Time	1507
OT Treatment Total Minutes	15 minutes
Additional Comments	15 min thera act & ADLs
**Subjective **
Subjective	Agreeable to OT.
Interpreter Services
Communication Needs	None
Precautions/Limitations
Precautions	Fall risk
Pain Assessment
Pain Assessment	Verbal Descriptor Scale (VDS)
VDS Pain Score	Moderate Pain (Pain Level 4-6)
Pain Type	Acute pain
Pain Location	Ankle
Pain Orientation	Right
Pain Radiating Towards	R heel
Pain Frequency	Intermittent
Pain Aggravating Factors	Walking
Pain Interventions	RN Notified;Rest
Response to Interventions	decreased pain at rest
Functional Assessment
Supine to Sit	Independent
Sit to Supine	Independent
Sit to Stand	Independent
Stand to Sit	Independent
Bed to Chair	Modified Independent (extra time/device/safety)
(FWW)
Chair to Bed	Modified Independent (extra time/device/safety)
(FWW)
Commode/Toilet Transfer	Modified Independent (extra time/device/safety)
(FWW)
Occupational Function
ADL/IADL (Y)	ADL
Self Feeding	Independent
Hygiene/Grooming	Independent
Dressing UE	Independent
Toileting/Pericare	Independent
Safety Awareness
Safety awareness with ADL/IADL	Good
Safety awareness with functional mobility	Good
Balance
Sitting Static	Good
Sitting Dynamic	Good
Standing Static	Good w/AD
Standing Dynamic	Good w/AD
**Therapeutic Activity **
Therapeutic activities	functional mobility w/ and w/o FWW; functional transfers
**Assessment/ Plan **
Assessment	Pt able to ambulate in room w/ good balance, but for longer distance requires using FWW for support d/t increased pain in R ankle and heel. She ambulated ~100 ft in hallway MOD I using FWW. Reports increased pain and return to chair. Pt able to complete toileting task by herself. Reports needing some help to don R socks d/t woundcare.
Plan	Continue w/ POC.
Plan of Care Frequency/Duration	1 time/day;5 times/week;2 weeks
Short Term Goals
Pt. Will Perform Toileting	Yes
Level of Assistance	Modified Independent (extra time/device/safety);Met
Location	Toilet
Pt. Will Perform Transfers	Yes
Level of Assistance	Modified Independent (extra time/device/safety);Met
Location	Toilet
Assistive Device/Equipment	Walker
OT Discharge Recommendations
OT Discharge Dispo Recommendations	Consult with CM/SW for Placement
OT Discharge Assistive Device Needs	FWW - front wheeled walker
OT Safety Concerns	homelss; pain in R foot

Patient returned/left seated on chair with:
[x] All needs within reach
**
[x]** RN updated on patient's status.
[ ] Other:

Education Provided:
- **Learner: ** [x]Patient** [ ]**Caregiver **[ ]**Family **[ ]** Other:
- **Barriers to Learning:** [x]None [ ]Language [ ]Literacy [ ]Vision [ ]Memory [ ] Hearing [ ]Other:
- **Learning Style:** [x]Verbal [ ]Written [ ] Demonstration [ ] Other:

[table#3]:
	***NFT**	**PS**	**VU**	**NVA**	**RD**	**RE**
Role of Occupational Therapy	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Using Nursing Call Light	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Fall Precautions	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Adaptive Equipment / Techniques	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Home Exercise Program	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Positioning/Mobility	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Therapeutic Exercise	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
Other:** **	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
***NFT** = needs further teaching** , PS**= requires practice/supervision, **VU**=verbalized understanding,
NVA=non-verbal acknowledgement; **RD**=return demonstration, **RE**=Refused Education

**Functional Goals: **Short Term Goals
Short Term Goals Due By: 02/03/24
Pt. Will Perform LE Dressing: Yes
Level of Assist: Modified Independent (extra time/device/safety), Met
Location: EOB
Assistive Device/Equipment: Walker
Pt. Will Perform Toileting: (P) Yes
Level of Assistance: (P) Modified Independent (extra time/device/safety), Met
Location: (P) Toilet
Assistive Device/Equipment: Walker
Pt. Will Perform Transfers: (P) Yes
Level of Assistance: (P) Modified Independent (extra time/device/safety), Met
Location: (P) Toilet
Assistive Device/Equipment: (P) Walker

[table#4]:
Plan

Continue with Plan of Care,

[table#5]:
PT/OT/SLP Recommendations

[table#6]:
Flowsheet Row	Most Recent Value
PT Dispo Recommendations	Consult with CM/SW for Placement filed at 02/02/2024 1238
PT Assistive Device Needs	FWW filed at 02/02/2024 1238
PT Safety Concerns	fall risk, homeless, concerns for self care filed at 02/02/2024 1238
OT Dispo Recommendations	Consult with CM/SW for Placement (P) filed at 02/02/2024 1507
OT Assistive Device Needs	FWW - front wheeled walker (P) filed at 02/02/2024 1507
OT Safety Concerns	homelss, pain in R foot (P) filed at 02/02/2024 1507
SLP Safety Concerns	Aspiration Precautions filed at 01/15/2024 1022
SLP Discharge Summary	Dysphagia 3/all liquids filed at 01/15/2024 1022
SLP Discharge Dispo Recommendation	Home filed at 01/15/2024 1022

[table#7]:

Thank you for including Inpatient Rehabilitation in the care of Elizabeth Tina Petruccio.

Elaheh Sobhani, OT
2/2/2024

Electronically signed by Elaheh S at 02/02/2024 3:56 PM PST

PT Treatment Note - Tania R - 02/02/2024 12:38 PM PST

Formatting of this note might be different from the original.

[table#1]:
| ***Physical Therapy - Treatment Note ***
***Scripps Mercy Hospital *** |
| --- |

[image:untitled image]
** **
Patient Name: Elizabeth Tina Petruccio
Admission Date: 1/12/2024
Treatment Date: 2/2/2024
Code Status: Full Code
Unit/Room: 0426/01

[table#2]:
SUBJECTIVE

History of Present Illness: Elizabeth Tina Petruccio is a 59 y.o. female who presents to Mercy Hospital with The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

Past Medical History: Elizabeth has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).
Past Surgical Intervention: Elizabeth has no past surgical history on file.

**Medical Status Update: ** n/a
Subjective: RN in agreement with PT Treatment, received pt in sitting at bed side chair and agreeab
le to therapy upon arrival. Following the treatment session, Elizabeth Tina Petruccio was left sitting at bed side chair with all needs within reach. RN is aware of pt status and therapy outcomes.

[table#3]:
OBJECTIVE

Relevant Lines/Drains:
[table#4]:
Patient Devices

[table#5]:
| | [table#5.1]
[table#5.2]
[table#5.3] |
| --- | --- |

[table#5.1]:
Active Devices

[table#5.2]:
	Name	Placement date	Placement time	Site	Days
	PICC Single Lumen 01/30/24 Right Basilic	01/30/24	1601	Basilic	3
	External Urinary Device	—	—	—	20

[table#5.3]:

[table#6]:

Therapist PPE Use: Surgical Mask

** Education Provided:**
- **Barriers to Learning:** [x]None [ ]Language [ ]Literacy [ ]Sight [ ]Memory [ ]Other:
- **Learning Style:** [x]Verbal [ ]Written [x]Demonstration [ ]Tactile [ ]Other:
- **Learner: **[x]Patient** [ ]**Caregiver **[ ]**Family **[ ]**Other:

[table#7]:
	NFT	**PS**	**VU**	**RD**
Role of Physical Therapy	[ ]	[ ]	[x]	[ ]
Call Light Use	[ ]	[ ]	[x]	[ ]
Fall Precautions	[ ]	[ ]	[x]	[ ]
Positioning/Mobility	[ ]	[x]	[ ]	[ ]
Assistive Device	[ ]	[x]	[ ]	[ ]
Safety with activity	[ ]	[x]	[ ]	[ ]
Therapeutic Exercise	[ ]	[ ]	[ ]	[ ]
Home Exercise Program	**[ ]**	**[ ]**	**[ ]**	**[ ]**
Other:** **	[ ]	[ ]	[ ]	[ ]
NFT = needs further teaching, **PS**= requires practice/supervision, **VU**=verbalized understanding, **RD**=return demonstration

[table#8]:
	02/02/24 1238
PT Treatment	
PT Treat Start Time	1214
PT Treat End Time	1238
PT Treatment Total minutes	24 minutes
Additional Comments	RN ok'd; GT tx2
**Subjective **	
Subjective	Agreeable to PT
Interpreter Services	
Communication Needs	None
Precautions/Limitations	
Precautions	Fall risk
Pain Assessment	
Pain Assessment	Verbal Descriptor Scale (VDS)
VDS Pain Score	Mild Pain (Pain Level 1-3)
Pain Type	Acute pain
Pain Location	Foot
Pain Orientation	Right
Pain Onset	Ongoing
Effect of Pain on Daily Activities	Activity
Pain Aggravating Factors	Walking
Pain Alleviating Factors	rest
Pain Interventions	RN Notified;Rest
Response to Interventions	effective
Vitals/Summary	
Time of Reading	During treatment
Patient Position	Sitting
SpO2	97 %
Oxygen Therapy	None (Room air)
Balance	
Sitting Static	Good
Sitting Dynamic	Good
Standing Static	Good -
Standing Dynamic	Good -
Bed Mobility Training	
Supine to Sit	
(NT - pt sitting at bedside chair)
Sit to Supine	
(NT - pt siting at bedside chair)
Transfers	
Sit to Stand	Independent
Stand to Sit	Independent
Bed to Chair	Supervision
Chair to Bed	Supervision

| Commode/Toilet Transfer | Stand by Assist |
| **Gait Training** |
| Gait Assistance | Supervision |
| Gait Distance (ft) | 100 ft
(25 x4 outside of room) |
| Gait Assistive Device | FWW
(vc for FWW managment and safety.) |
| Gait Patterns | Antalgic
(short stride length 2/2 pain in RLE) |
| Stairs Assistive Device | FWW |
| **Safety Awareness** |
| Safety Awareness with Functional Mobility | Good |
| **Neuro Assessment** |
| Alertness | Intact |
| **Assessment/Plan** |
Assessment	see note
Plan	progress gt AD to SPC, reduce POC fequency
Plan of Care Frequency/Duration	3 times/week
Short Term Goals
Short Term Goals Due By	02/16/24
Pt. Will Perform Transfer	Met
Pt. Will Perform Gait	Met;New
Goal
(100 ft with SPC or no AD)
6 Clicks Basic Mobility PT
Turning from your back to your side while in a flat bed without using bedrails?	4
Moving from lying on your back to sitting on the side of a flat bed without using bedrails?	4
Moving to and from a bed to a chair (including a wheelchair)?	4
Standing up from a chair using your arms (e.g., wheelchair or bedside chair)?	4
Walking in hospital room?	3
Climbing 3-5 steps with a railing?	3
PT 6 Clicks Score	22
PT Discharge Recommendations
PT Discharge Dispo Recommendations	Consult with CM/SW for Placement
PT Discharge Assistive Device Needs	FWW
PT Safety Concerns	fall risk, homeless, concerns for self care

**Treatment summary: **Therapy session consisted of transfers, gait training, and pt education. Pt received gait training with FWW. Requires vc for proper AD management and safety while ambulating. Received cues for even stride length and heel-toe contact during ambulation to minimize pain in RLEE. Pt educated on how to ween off FWW and progress AD to SPC next session.

[table#9]:
ASSESSMENT

Assessment: Elizabeth Tina Petruccio was seen for follow up Physical Therapy session and is showing good progression towards their functional goals demonstrated by improved activity tolerance. Pt presents with deficits including decreased standing balance and RLE pain contributing to her risk for falls. Pt's has met some of her functional goals, therefore they have been updated to reflect her progress. POC frequency has decreased given her improvement in independence. Elizabeth Tina Petruccio will benefit from continued inpatient physical therapy to maximize functional return. Pt has been recmomeded to consult with CM/SW for placemnt given concerns for self care and fall risk.

**Functional Goals: **Short Term Goals
Short Term Goals Due By: 02/16/24
Short Term Goal (Y): Yes
Pt. Will Perform Bed Mobility: Independent
Pt. Will Perform Transfer : Met
Pt. Will Perform Gait: Met, New Goal (100 ft with SPC or no AD)
Pt. Will Go Up / Down Stairs: Modified Independent (extra time/device/safety((LRAD, curb management for community access)

**Recommendations to Nursing: **Ambulate multiple times daily with nursing assistance, utilize FWW

[table#10]:
| **DC Recommendations: **PT Discharge Recommendations
PT Discharge Dispo Recommendations: Consult with CM/SW for Placement
PT Discharge Assistive Device Needs: Front Wheeled Walker
PT Safety Concerns: fall risk, homeless, concerns for self care |
| --- |

[table#11]:
PLAN

**Plan: **Continue PT treatment as appropriate and tolerated per Plan of Care while admitted to Hospital. **Focus on gait with SPC at next treatment. **

Thank you for including Inpatient Rehabilitation in the care of Elizabeth Tina Petruccio. Please contact Physical Therapy for any needs or concerns at extension x8645.

Tania Rojas-Gonzalez, PT
2/2/2024
** **

Electronically signed by Tania R at 02/02/2024 2:34 PM PST

Plan of Care - Your Scripps Health Team - 02/02/2024 11:02 AM PST
Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Alert and oriented x4,afebrile.@ 98% saturation on room .On respiratory treatments PRN,no distress.Ambulatory to the bathroom on her own.Instructed on safety.Reminded on fluid restriction as ordered.Na 130,MD aware.

Key findings related to barriers to transfer/discharge:Medical clearance,awaits placement

Patient/Family needs and concerns:Same as above

Electronically signed by Your Scripps Health Team at 02/02/2024 11:06 AM PST

Plan of Care - Nurse Esther N - 02/01/2024 11:06 PM PST
Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt is continuing with plan of care. Pt is A/Ox4, VSS, on RA while awake, 2LNC while asleep for desatting. Pt is forgetful. Pt reports severe neck pain, adequately relieved by PRN dilaudid. PRN vistaril and melatonin given for anxiety and insomnia. Fall precautions, call light within reach.

**Key findings related to barriers to transfer/discharg
e: **medical clearance, SNF placement

Patient/Family needs and concerns: updates PRN

Electronically signed by Nurse Esther N at 02/01/2024 11:08 PM PST

PT Treatment Note - Your Scripps Health Team - 02/01/2024 11:44 AM PST
Formatting of this note might be different from the original.
Physical Therapy
[table#1]:
| ***Physical Therapy - Treatment Note***
***Scripps Mercy Hospital *** |
| --- |

[image]

Patient Name: Elizabeth Tina Petruccio
Admission Date: 1/12/2024
Eval Date: 2/1/2024
Code Status: Full Code
Unit/Room: 0426/01

Social History
Type of Residence: Shelter
Prior Level of Function Mobility : Independent, Modified Independent (extra time/device/safety)
Assistive Device : (none)
Living Arrangements: Alone
Communication: WNL
Prior Level of Function

Functional Mobility: Independent
ADLs: Independent

[table#2]:
SUBJECTIVE

Primary Diagnosis: The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

[table#3]:
OBJECTIVE

Relevant Lines/Drains:
[table#4]:
Patient Devices

[table#5]:
| | [table#5.1]
[table#5.2]
[table#5.3] |
| --- | --- |

[table#5.1]:
Active Devices

[table#5.2]:
	Name	Placement date	Placement time	Site	Days
	PICC Single Lumen 01/30/24 Right Basilic	01/30/24	1601	Basilic	2
	External Urinary Device	—	—	—	19

[table#5.3]:

[table#6]:

[table#7]:
	02/01/24 1129
PT Treatment
PT Treat Start Time	1050
PT Treat End Time	1117
PT Treatment Total minutes	27 minutes
Additional Comments	therex17min, gait trng10min. RN Estela cleared pt for Rx.
(BTB at completion of Rx, call bell/needs in reach)
**Subjective **
Subjective	Agrees to PT.
Interpreter Services
Communication Needs	None
Pain Assessment
Pain Assessment	0-10
Pain Score	4
Pain Location	Neck
Pain Orientation
(plus intermittent LBP)
Pain Interventions	Medication (See MAR)
(indicated she was avoiding asking for pain meds unless really necessary)
Response to Interventions
(indicated pain "not too much right now")
Vitals/Summary
Heart Rate	104
SpO2	94 %

| Oxygen Therapy | None (Room air) |
| **Balance** |
Sitting Static	Good
Sitting Dynamic	Good w/UE
Standing Static	Fair +
Standing Dynamic	Fair + w/AD
Bed Mobility Training
Sit to Supine	Independent
Transfers
Sit to Stand	Independent
Stand to Sit	Independent
Chair to Bed	Independent
Gait Training
Gait Assistance	Supervision
Gait Distance (ft)	130 ft
Gait Assistive Device	FWW
Gait Patterns	Antalgic
Safety Awareness
Safety Awareness with Functional Mobility	Good
Therapeutic Exercises
Hip Abd/Add	seated isometric ADD sets 5x5secB
Standing Hip Abd	5ea
Supine Hip ER	seated isometric ABD sets 5x5secB
Other	hip ext standing 5ea; trial hip flexor stretch in supine LE over edge w/opposite KTC for lumbar stab--back pain worsened therefore DCd (had c/o groin tightness)
**Other **
Other (Comment)	Pt ed re fwd lean intermittently to alleviate LBP while in standing/walking. RD. Ed to avoid exs/moves that exacerbate her typical pain. She CU. Ed re amb w/nsg a couple of times/day in addition to therapies. She CU
(Ed re using support through UEs on walker to permit better positioning of R foot during gt vs amb on lateral border of foot, and with time aim is for more normal foot mechanics as gout pain subsides, to avoid secondary problems from abnormal gt mech.)
PT Discharge Recommendations
PT Discha
rge Dispo Recommendations	SNF
(per PT 1/23, pending progress)
PT Discharge Assistive Device Needs	FWW
PT Safety Concerns	fall risk w/o walker

[table#8]:
ASSESSMENT

**Assessment: ** Good participation. Less assistance needed and greater distance tolerated with gait. Pain seemed reasonably controlled.

**Functional Goals: **Short Term Goals
Short Term Goals Due By: 02/04/24
Short Term Goal (Y): Yes
Pt. Will Perform Bed Mobility: Independent
Pt. Will Perform Transfer : Modified Independent (extra time/device/safety) (LRAD)
Pt. Will Perform Gait: Modified Independent (extra time/device/safety) (LRAD, 100 feet)
Pt. Will Go Up / Down Stairs: Modified Independent (extra time/device/safety((LRAD, curb management for community access)

**Recommendations to Nursing: **Ambulate multiple times daily with nursing assistance, utilize FWW

[table#9]:
PLAN

Plan: Continue Physical Therapy as tolerated and appropriate per plan of care. ** Focus on gentle exs in addition to gait progression at next treatment session. FWW advised at this time 2* gout R foot/ankle and spinal issues**

Thank you for including Inpatient Rehabilitation in the care of Elizabeth.

Elaine Dorsey, PTA
2/1/2024

Electronically signed by Your Scripps Health Team at 02/01/2024 11:48 AM PST

Plan of Care - Your Scripps Health Team - 02/01/2024 10:21 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Alert and oriented x4,afebrile
@ 95% saturation on room air.No complaints of pain,no respiratory discomfort.Ambulatory to the bathroom on her own.Reminded on fluid restriction and I/O.
Key findings related to barriers to transfer/discharge: Medical clearance,placement

Patient/Family needs and concerns:Same as above

Electronically signed by Your Scripps Health Team at 02/01/2024 10:27 AM PST

Plan of Care - Nurse Samantha A - 02/01/2024 12:21 AM PST
Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: A&Ox4 verbally responsive, resting comfortably in bed. PRN for anxiety and pain effective on follow up. Continuing to benefit from redirection, ambulation, and repositioning with episodes of anxiety. Verbalizes understanding with current plan of care, safety precautions, and call light to be used when needing assistance.

**Key findings related to barriers to transfer/discharge: **Medical clearance.

Patient/Family needs and concerns: Addressed through shift.

Electronically signed by Nurse Samantha A at 02/01/2024 12:42 AM PST

OT Evaluation - Elaheh S - 01/31/2024 12:25 PM PST
Formatting of this note might be different from the original.
** Occupational Therapy**

[image]

Patient Name: Elizabeth Tina Petruccio
Admission Date: 1/12/2024
Eval Date: 1/31/2024
Unit/Room: 0426/01
Code Status: Full Code

**Primary Diagnosis: **The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

History of Present Illness: Elizabeth Tina Petruccio is a 59 y.o. female who presents to Scripps Mercy Chula Vista with

[table#1]:
Chief Complaint
Patient presents with
•	Shortness of Breath
		Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB

Past Medical History:
[table#2]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Past Surgical Intervention: Elizabeth has no past surgical history on file.
Pertinent Social History: Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

[table#3]:
Subjective / Objective / Assessment

[table#4]:
| | **01/31/24 1225** |

| --- | --- |
| **OT Treatment** |
| OT Treatment Start Time | 1210 |
| OT Treatment End Time | 1225 |
| OT Treatment Total Min
utes | 15 minutes |
| Additional Comments | 15 min thera act & ADLs. |
| **Subjective ** |
| Subjective | RN cleared pt for OT; pt agreeable. |
| **Interpreter Services** |
| Communication Needs | None |
| **Precautions/Limitations** |
| Precautions | Fall risk |
| **Pain Assessment** |
Pain Assessment	Verbal Descriptor Scale (VDS)
VDS Pain Score	Mild Pain (Pain Level 1-3)
Pain Type	Acute pain
Pain Location	Foot
Pain Orientation	Right
Pain Descriptors	Aching
Pain Frequency	Intermittent
Pain Aggravating Factors	Walking
Pain Interventions	Pre-Medicated by RN;Rest
Functional Assessment
Sit to Stand	Modified Independent (extra time/device/safety)
Stand to Sit	Modified Independent (extra time/device/safety)
Commode/Toilet Transfer	Stand by Assist
Occupational Function
ADL/IADL (Y)	ADL
Self Feeding	Independent
Hygiene/Grooming	Independent
Oral Care	Denture care
Dressing UE	Independent
Dressing LE	Modified Independent (extra time/ device/ safety)
Toileting/Pericare	Stand-by Assist
Safety Awareness
Safety awareness with ADL/IADL	Good
Safety awareness with functional mobility	Good
Balance
Sitting Static	Good
Sitting Dynamic	Good w/UE
Standing Static	Fair +
Standing Dynamic	Fair
**Therapeutic Activity **
Therapeutic activities	functional mobility; functional transfer training; balance training
**Assessment/ Plan **
Assessment	Pt shows progress toward therapy goals. Demos improved balance and activity tolerance. Reports decreased overall pain in R foot and was able to ambulate ~120 w/o FWW w/ SBA. She currently presents close to baseline for ADLs. OT will continue seeing pt to promote safety and ind. prior to d/c.
Plan	Continue w/ POC.
Plan of Care Frequency/Duration	1 time/day;5 times/week;2 weeks
Short Term Goals
Pt. Will Perform LE Dressing	Yes
Level of Assist	Modified Independent (extra time/device/safety);Met
Location	EOB
6 Clicks Daily Activity OT
Putting on and taking off regular lower body clothing?	4
Bathing (including washing, rinsing, drying?)	3
Toileting, which includes using toilet, bedpan, or urinal?	3
Putting on and taking off regular upper body clothing?	4
Taking care of personal grooming such as brushing teeth?	4
Eating meals?	4
OT 6 Clicks Score	22
OT Discharge Recommendations
OT Discharge Dispo Recommendations	Consult with CM/SW for Placement
OT Discharge Assistive Device Needs	FWW - front wheeled walker
OT Safety Concerns	homelss

Patient returned/left seated on chair with:
[x] All needs within reach

[x] RN updated on patient's status.
[ ] Other:

Education Provided:
- **Learner: ** [x]Patient** [ ]**Caregiver **[ ]**Family **[ ]** Other:
- **Barriers to Learning:** [x]None [ ]Language [ ]Literacy [ ]Vision [ ]Memory [ ] Hearing [ ]Other:
- **Learning Style:** [x]Verbal [ ]Written [x] Demonstration [ ] Other:

[table#5]:
	***NFT**	**PS**	**VU**	**NVA**	**RD**	**RE**
Role of Occupational Therapy	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Using Nursing Call Light	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Fall Precautions	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Adaptive Equipment / Techniques	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Home Exercise Program	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Positioning/Mobility	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
Therapeutic Exercise	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
Other:** **	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
***NFT** = needs further teaching** , PS**= requires practice/supervision, **VU**=verbalized understanding,
NVA=non-verbal acknowledgement; **R
D**=return demonstration, **RE**=Refused Education

Please see future OT notes for updates for discharge recommendations and equipment needed.

[table#6]:
Plan

Occupational Therapy treatment as tolerated until functional goals met or patient is discharged from facility.

Thank you,
Elaheh Sobhani, OTR/L

Electronically signed by Elaheh S at 01/31/2024 2:21 PM PST

Plan of Care - Your Scripps Health Team - 01/31/2024 11:05 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt is stable, AO X4, follows commands, pt is ST/NSR with PVCs on the monitor, on RA, pt denies any chest pain or SOB, pt is able to tolerate po diet and medications, pt is on po and IV antibiotics. Continue to monitor pt, labs and VS.

Key findings related to barriers to transfer/discharge: Medical clearance

Patient/Family needs and concerns: none at this time

Electronically signed by Your Scripps Health Team at 01/31/2024 11:08 AM PST

Plan of Care - Nurse Samantha A - 01/31/2024 1:48 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: A&Ox4 verbally responsive, resting comfortably in bed through shift. PRN for sleep and pain to r. Hip effective on follow up. Assisted with repositioning, redirection, and reassurance through shift. Patient verbalizes understanding with current plan of care, safety precautions, and call light to be used when needing assistance. Continuing IV ABT treatment, RUE PICC in place.

**Key findings related to barriers to transfer/discharge: **Medical clearance.

Patient/Family needs and concerns: Addressed through shift.

Electronically signed by Nurse Samantha A at 01/31/2024 1:50 AM PST

Plan of Care - Your Scripps Health Team - 01/30/2024 5:05 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Patient resting on the chair, alert, oriented x4, no complaint of pain, PICC line just placed in IR. Seen by Pulmonology this morning to evaluate the need for another thoracentesis, patient did not need to have the procedure done per Dr. Jones. Plan is for IV antibiotic treatment for a couple of weeks due to osteomyelitis of the spine.

**Key findings related to barriers to transfer/discharge: **

Patient/Family needs and concerns:

Electronically signed by Your Scripps Health Team at 01/30/2024 5:10 PM PST

Perioperative Nursing Note - Your Scripps Health Team - 01/30/2024 3:45 PM PST

Formatting of this note might be different from the original.
Pt stable no distress

Electronically signed by Your Scripps Health Team at 01/30/2024 3:45 PM PST

Plan of Care - Your Scripps Health Team - 01/29/2024 7:45 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: @1900- in bed asleep; desaturates to 80's O2 sats; placed on 2L nasal cannula; no acute distress noted; denies any pain.R

Received Dilaudid 0.5 mg IV and Naproxen 500 mg po for headache and low back pain tonight.
Patient was able to use IS reaching 1250.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: none

Electronically signed by Your Scripps Health Team at 01/29/2024 7:47 PM PST
Electronically signed by Your Scripps Health Team at 01/30/2024 6:41 AM PST

OT Patient Not Seen Note - Candice D - 01/29/2024 2:39 PM PST

Formatting of this note might be different from the original.
[table#1]:
| ***Occupational Therapy - Patient Not Seen Notification***
***Scripps Mercy Hospital *** |
| --- |

[image]

Patient Not Seen Notification

Patient Name: Elizabeth Tina Petruccio
Unit/Room: 0426/01
**Primary Diagnosis: **The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

**Date: **1/29/2024

**Reason For Deferral: **
[ ] Patient Refused
[ ] Family Deferred
[ ] Therapist Deferred
[ ] RN Deferred
[ ] MD Deferred
[ ] Medical Contraindication
[ ] Awaiting Consultation
[ ] Pt Unable to Participate
[x] Pt Unavailable for Treatment
[ ] Scheduling Conflict
[ ] Patient At or Pending Surgery
[ ] Other:

**Comment: **Pt currently in IR for B LE US and CT. OT to reattempt as schedule and pt status permit.

For questions or concerns, please contact **Rehab at x7484 **or contact the assigned therapist on the daily **Treatment Team**.

Candice Dumais, OT
1/29/2024

Electronically signed by Candice D at 01/29/2024 2:40 PM PST

Plan of Care - Your Scripps Health Team - 01/29/2024 1:40 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt. A&Ox4, VSS, SpO2 96% on RA, diuretics continued. Tolerating diet well, up to bathroo
m, voiding with good output.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: none

Electronically signed by Your Scripps Health Team at 01/29/2024 1:47 PM PST

Plan of Care - Your Scripps Health Team - 01/28/2024 7:48 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: @1900-Patient received lying in bed without any acute distress; denies any pain; plan of care discussed. @2100-received Ambien 5mg po for sleep and Naproxen 500 mg for pain.
@0050- Received Dilaudid 0.5 mg IV for back pain. @0343-(+) anxious; Vistaril 25 mg po given;
(+) nausea; Zofran 4 mg IV given.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: none

Electronically signed by Your Scripps Health Team at 01/28/2024 7:49 PM PST
Electronically signed by Your Scripps Health Team at 01/29/2024 4:21 AM PST

Plan of Care - Your Scripps Health Team - 01/28/2024 9:27 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: aox4, denies chest pains, no sob, on 2 liters/nc sats 92%. Pain management. Diuretics on board.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: none

Electronically signed by Your Scripps Health Team at 01/28/2024 9:28 AM PST

Plan of Care - Your Scripps Health Team - 01/28/2024 4:54 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: AOx4, ST on the monitor, VSS, generalized pain to back/body/joints that is moderately controlled with PRNs.

**Key findings related to barriers to transfer/discharge: **Medical clearance.

Patient/Family needs and concerns: Pt is upset that she cannot have the floors one dialysis chair, since that helps relieve her pain most. It was explained that we will need the chair for AM HD cases but pt is still upset.

Electronically signed by Your Scripps Health Team at 01/28/2024 5:02 AM PST

Plan of Care - Your Scripps Health Team - 01/27/2024 8:41 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: aox4, denies chest pains, no sob, on RA sats 92%. Pain management. Informed MD Khayyat of latest hgb 7.6 hct 24.3, NA 129.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: none

Electronically signed by Your Scripps Health Team at 01/27/2024 8:44 AM PST
Electronically signed by Your Scripps Health Team at 01/27/2024 9:46 AM PST

Plan of Care - Your Scripps Health Team - 01/27/2024 3:05 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: A/O x 4, calm, ST on tele, HR- low 100's, no c/o chest pain, no SOB, O2 sat 98% on RA when awake, O2 sat went down to high 80's when patient asleep, put on O2 @ 1 LPM NC, O2 sat went back up to 95%. Afebrile.
**Key findings related to barriers to transfer/discharge: **
Patient/Family needs and concerns:

Electronically signed by Your Scripps Health Team at 01/27/2024 3:14 AM PST

Plan of Care - Your Scripps Health Team - 01/26/2024 3:54 PM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: Patient is alert and oriented x 4. ST on telemetry, on RA, denies chest pain. MRI of spine completed this AM. Managing pain with PRN medications, with effect. Safety precautions in place.

**Key findings related to barriers to transfer/discharge: **Medical clearance.

Patient/Family needs and concerns: None.

Electronically signed by Your Scripps Health Team at 01/26/2024 4:00 PM PST

Plan of Care - Your Scripps Health Team - 01/26/2024 2:45 PM PST

Formatting of this note might be different from the original.

**Clinical Nutrition Monitor Note **

Recommendations to Provider:
1. Continue Regular/dental soft diet w/ 1500 mL fluid restriction per MD
2. Continue multivitamin

**Nutrition Diagnosis (PES statement): **
Inadequate oral Intake* *related to previous mechanical ventilation, now extubated with PO diet as evidenced by average PO intake greater than 75% of meals, now meeting estimated kcal needs. *-ongoing/improving *

[table#1]:
|
|
| --- |
| [table#1.1] |

[table#1.1]:
Dietary Orders
(From admission, onward)

[table#2]:
|
|
| --- |

[table#3]:
| | [table#3.1]
[table#3.2] |
| --- | --- |

[table#3.1]:
	Start			Ordered
	01/22/24 1101		[table#3.1.1]	01/22/24 1100

[table#3.1.1]:
Combination Diet Regular; 1500 mL; Dental Soft Diet effective now
[table#3.1.1.1]

[table#3.1.1.1]:

Question	Answer	Comment
Please Select Base Diet:	Regular	
Fluid Restriction - Oral (mL/day):	1500 mL	
Food Consistency:	Dental Soft	

[table#3.2]:

[table#4]:

Food Allergies: NKFA, lactose intolerance noted

Nutritionally significant labs: Na+ 131

Nutritionally significant medications: IV antibiotics, magic mouthwash, multivitamin, senokot-S

Skin: intact

**Anthropometrics: **
Height: 167.6 cm (5' 6"), Weight: 68 kg (149 lb 14.6 oz), BMI (Calculated): 24.2

Weight trend this admission: [ ] Loss[ ] Gain [x] Stable [ ] Requested new weight
Weight change assessment:
[table#5]:
	Date			
	Height	Weight	BMI (Calculated)	Weight Method
1/12/2024	170.2 cm	65.1	22.5 kg/m2	
1/22/2024		65.318	22.5 kg/m2	
1/25/2024	167.6 cm	68	24.2 kg/m2	Bed Scale

**Estimated needs: **
Weight Used for Equation Calculations: 63.6 kg (140 lb 3.4 oz), Describe Weight Used: Actual
Total Daily Calorie Estimated Needs: 1600-1900 (25-30 kcal/kg) kcal/day
Total Daily Protein Estimated Needs : 65-75 (1-1.2 g/kg) g/day
Total Daily Fluid Estimated Needs: 1600-1900 mL/day

Nutrition Related Summary since last visit: Unable to reach pt over the phone for nutrition interview. Average PO intake consistently greater than 75% meals since 1/23. Pt ordering 3 nutritionally balanced meals per day. Noted last BM 1/23, senokot-S ordered and given. Na+ remains low on fluid restriction.

**Is patient meeting estimating needs?: **[x] Yes [ ] No
Are current nutrition related orders adequate and appropriate?: [x] Yes [ ] No
Change recommended: n/a

**Plan of Care Discussion: **RN

Nutrition Level of Care: Moderate (LC2)

Monitoring/Evaluation: PO intake, weight trend, BM, Na+

Goals: PO intake to meet GT 75% estimated needs ~ *met/ongoing*

Kelley S., RD, CNSC
Clinical Nutrition
312-7176

Electronically signed by Your Scripps Health Team at 01/26/2024 3:10 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt continues with plan of care. Pt is A&O x4, calm, pleasant, and cooperative with all nursing interventions. At the beginning of the shift, pt requested a sleep aid due to insomnia; PRN Ambien was given with successful results. Pt also c/o severe R hip pain; PRN IVP Dilaudid was given with effective pain relief. Pt is now longer on PO antibiotics but one dose of Augmentin was given during my shift, before it was discontinued. Pt now started on IV antibiotics; one dose of IV Rocephin and Vancomycin were given. Pt is currently on 1L supplemental oxygen via nc due to multiple desaturation events while sleeping, and showing oxygen saturation readings in the mid to high 90's. Around 0100, pt was awakened from sleep c/o a moderate headache; PRN Naproxen was given. Pt's R later back thoracentesis dressing remains clean, dry, and intact with no signs of drainage. Pt midline IV access remains patient, dressing is clean, dry, and intact, and all connections have been checked and tightened. Pt's telemetry strip revealed NSR with a QTC in the yellow zone; pharmacy is aware. Pt's VSS, denies chest pain, or any discomfort at this time, and is not in acute distress. Pt has been oriented to the unit, and displays appropriate safety awareness and all safety measures are in place; bed in the lowest position, bedside table within reach, bed alarm on, and call light at arm's length. Pt is currently resting in bed, comfortably sleeping.

**Key findings related to barriers to transfer/discharge: **SNF placement. Medical clearance.

Patient/Family needs and concerns: None at this time.

Electronically signed by Your Scripps Health Team at 01/26/2024 3:30 AM PST

Procedure Report - Christopher Jones, MD - 01/25/2024 12:04 PM PST

Formatting of this note might be different from the original.
Procedure: Diagnostic and therapeutic paracentesis/thoracentesis.

Preoperative Diagnoses:
Parapneumonic effusion

Postoperative Diagnoses:
Para
pneumonic effusion, drained

Procedure in Detail:
Informed consent was obtained from the patient after a discussion of the risks, benefits, and alternatives. The right hemithorax was examined under ultrasound, and a large pocket of fluid was identified. This area was prepared with chlorhexidine and draped in a sterile fashion. All operators used sterile technique including sterile gloves. Analgesia was obtained using the infiltration of 1% lidocaine into the subcutaneous tissue. A small nick was made with the scalpel. Next, a 8 French centesis catheter needle was inserted into the abdomen/hemithorax and serosanguineous was obtained. The catheter was advanced and the needle was removed.

A total of 300cc of fluid was removed from the patient. Following this, the needle was removed in standard fashion, and a bandage was applied.

The patient tolerated the procedure well. There were no immediate complications. The fluid was sent for appropriate studies.

EBL: <10cc, fluid and the procedure was more slightly more sanguinous than serosanguineous, likely due to significant infection in the chest and recent chest tube placement

Images below are of pre and post procedure

[image]

[image]

[image]

Electronically signed by Christopher Jones, MD at 01/25/2024 12:06 PM PST

Plan of Care - Your Scripps Health Team - 01/25/2024 10:03 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt alert and oriented x4. VSS. Upon beginning of shift pt was on RA but shortly after desating to mid 80's. Pt could not recover until placed back on oxygen. Currently on 1LNC. Will continue to try to wean. Pt given prn naproxen for headache. Thoracentesis planned today at bedside. Pt complains of neck pain when she sleeps. MD told pt we will do some imaging studies. Fall precautions in place. Continue with plan of care.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: None at this time.

Electronically signed by Your Scripps Health Team at 01/25/2024 10:06 AM PST

Plan of Care - Your Scripps Health Team - 01/24/2024 11:37 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt continues with plan of care. Pt is A&O x4, calm, pleasant, and cooperative with all nursing interventions. At the beginning of the shift, pt requested a sleep aid due to insomnia; PRN Melatonin was given with successful results. Pt continues on PO antibiotics; one dose of Augmentin was given during my shift. Pt is currently on 0.5L supplemental oxygen via nc and showing oxygen saturation readings in the mid to high 90's. Pt's RL flank chest tube excision dressing remains clean, dry, and intact with no signs of drainage. Pt midline IV access remains patient, dressing is clean, dry, and intact, and all connections have been checked and tightened. Pt's telemetry strip revealed NSR with a QTC in the yellow zone; pharmacy is aware. Pt's VSS, denies chest pain, or any discomfort at this time, and is not in acute distress. Pt has been oriented to the unit, and displays appropriate safety awareness and all safety measures are in place; bed in the lowest position, bedside table within reach, bed alarm on, and call light at arm's length. Pt is currently resting in bed, comfortably sleeping.

**Key findings related to barriers to transfer/discharge: **SNF placement. Medical clearance.

Patient/Family needs and concerns: None at this time.

Electronically signed by Your Scripps Health Team at 01/24/2024 11:42 PM PST
Electronically signed by Your Scripps Health Team at 01/25/2024 2:38 AM PST

Procedure Report - Christopher Jones, MD - 01/24/2024 1:30 PM PST

Formatting of this note might be different from the original.
Thoracic Ultrasound

Done for diagnostic purposes to evaluate for loculated pleural effusion given progression of effusion on chest x-ray

Patient was positioned in the left lateral decubitus position to best expose her right hemithorax. Real-time ultrasound was used to evaluate the area from the anterior chest to the mid axillary line to the posterior position. Notable that the anterior liver lung interface is well-preserved, but posteriorly inferior to the scapular border there was a pocket of fluid roughly 5 cm deep which would be amendable to percutaneous drainage. Given patient received Lovenox earlier this morning, will plan to hold Lovenox and proceed with procedu
re tomorrow. This was discussed with the patient and she was amenable to proceeding. Lovenox held.

[image]

[image]

[image]

Electronically signed by Christopher Jones, MD at 01/24/2024 1:32 PM PST

Plan of Care - Your Scripps Health Team - 01/24/2024 10:56 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt alert and oriented x 4; calm and cooperative with all nursing interventions. Vital signs stable, denies chest pain. Pt weaned down to 0.5L NC. Pt experienced some SOB, prn nebs administered. PRN medications used to alleviate pain and discomfort from mouth sores. Pt receiving PO abx. Pt uses hot/cold and packs and elevation for her discomfort on her R ankle. Fall precautions in place. Continue with plan of care.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: None at this time.

Electronically signed by Your Scripps Health Team at 01/24/2024 11:01 AM PST

Plan of Care - Your Scripps Health Team - 01/24/2024 1:32 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt continues with plan of care. Pt is A&O x4, calm, and cooperative with all nursing interventions. At the beginning of the shift, pt requested Lidocaine topical solution for her mouth lesions; medication was successfully applied using a toothette. Pt c/o moderate headache; PRN Naproxen was given with minimal pain relief. Pt continues experiencing severe RLE ankle pain, exacerbated by exercising bathroom privileges; PRN IV Dilaudid was given with effective results. Pt's RLE ankle remains elevated, and heat / cold packs are being applied intermittently. Pt continues on PO antibiotics; one dose of Augmentin was given during my shift. Pt is currently on 1L supplemental oxygen via nc due to sustained desaturation events while sleeping. Pt's RL flank chest tube excision dressing remains clean, dry, and intact with no signs of drainage. Pt midline IV access remains patient, dressing is clean, dry, and intact, and all connections have been checked and tightened. Pt's telemetry strip revealed NSR. Pt's VSS, denies chest pain, or any discomfort at this time, and is not in acute distress. Pt has been oriented to the unit, and displays appropriate safety awareness and all safety measures are in place; bed in the lowest position, bedside table within reach, bed alarm on, and call light at arm's length. Pt is currently resting in bed, comfortably sleeping.

**Key findings related to barriers to transfer/discharge: **SNF placement. Medical clearance.

Patient/Family needs and concerns: None at this time.

Electronically signed by Your Scripps Health Team at 01/24/2024 2:44 AM PST

OT Treatment Note - Elaheh S - 01/23/2024 3:49 PM PST

Formatting of this note might be different from the original.
** Occupational Therapy**
[image]

Patient Name: Elizabeth Tina Petruccio
Admission Date: 1/12/2024
Treatment Date: 1/23/2024
Code Status: Full Code
Unit/Room: 0426/01

**Diagnosis: **The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

Social History
(Retired) Type of Residence: Shelter
Assistive Device : (none)
Living Arrangements: Alone
Communication: WNL

Prior Level of Function
Functional Mobility: Independent
ADLs: Independent

[table#1]:
Subjective / Objective / Assessment

[table#2]:
	01/23/24 1549
OT Treatment
OT Treatment Start Time	1510

OT Treatment End Time	1547
OT Treatment Total Minutes	37 minutes
Additional Comments	37 ADLs and thera act
**Subjective **
Subjective	RN cleared pt for OT; pt agreeable.
Interpreter Services
Communication Needs	None
Precautions/Limitations
Precautions	Fall risk
Pain Assessment
Pain Assessment	Verbal Descriptor Scale (VDS)
VDS Pain Score	Severe Pain (Pain Level 7-10)
Pain Type	Acute pain
Pain Location	Ankle
Pain Orientation	Right
Pain Radiating Towards	R lower leg
P
ain Descriptors	Aching
Pain Frequency	Constant/continuous
Pain Aggravating Factors	Reposition/movement;Palpation/touch;Standing;Walking
Pain Interventions	Rest;RN Notified
Functional Assessment
Supine to Sit	Contact Guard Assist
Sit to Supine	Contact Guard Assist
Sit to Stand	Contact Guard Assist
Stand to Sit	Contact Guard Assist
Bed to Chair	Contact Guard Assist
(BSC)
Chair to Bed	Contact Guard Assist
Occupational Function
ADL/IADL (Y)	ADL
Self Feeding	Independent
Hygiene/Grooming	Set-up
Dressing UE	Minimal Assistance (Subject 75% or more)
Dressing LE	Moderate Assistance (Subject 50-74%)
Toileting/Pericare	Moderate Assistance (Subject 50-74%)
Safety Awareness
Safety awareness with ADL/IADL	Good
Safety awareness with functional mobility	Good
Balance
Sitting Static	Fair + w/UE
Sitting Dynamic	Fair - w/UE
Standing Static	Fair - w/AD
Standing Dynamic	Fair - w/AD
**Therapeutic Activity **
Therapeutic activities	bed mobility/supine<>sit; sit<>stand; functional transfer training
**Assessment/ Plan **
Assessment	Pt reports severe pain in R ankle and unable to ambulate w/ OT this pm. Agreeable to use BSC for toileting d/t unable to ambulate to bathroom. Pt continues to need Mod a for toileting and LB-related ADLs d/t pain and weakness. OT will continue working w/ pt to promote safety and IND prior to d/c.
Plan	Continue w/ POC.
Plan of Care Frequency/Duration	1 time/day;5 times/week;2 weeks
OT Discharge Recommendations
OT Discharge Dispo Recommendations	SNF
OT Safety Concerns	fall risk; homeless

Patient returned/left supine in bed with:
[x] All needs within reach
[x] RN updated on patient's status.
[ ] Other:

Education Provided:
- **Learner: ** [x]Patient** [ ]**Caregiver **[ ]**Family **[ ]** Other:
- **Barriers to Learning:** [x]None [ ]Language [ ]Literacy [ ]Vision [ ]Memory [ ] Hearing [ ]Other:
- **Learning Style:** [x]Verbal [ ]Written [x] Demonstration [ ] Other:

[table#3]:
	***NFT**	**PS**	**VU**	**NVA**	**RD**	**RE**
Role of Occupational Therapy	[ ]	[ ]	[x]	[ ]	[ ]	[ ]

Using Nursing Call Light	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Fall Precautions	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Adaptive Equipment / Techniques	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Home Exercise Program	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Positioning/Mobility	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Therapeutic Exercise	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
Other:** **	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
***NFT** = needs further teaching** , PS**= requires practice/supervision, **VU**=verbalized understanding,
NVA=non-verbal acknowledgement; **RD**=return demonstration, **RE**=Refused Education

**Functional Goals: **Short Term Goals
Short Term Goals Due By: 02/03/24
Pt. Will Perform LE Dressing: Yes
Level of Assist: Modified Independent (extra time/device/safety)
Location: EOB
Assistive Device/Equipment: Walker
Pt. Will Perform Toileting: Yes
Level of Assistance: Modified Independent (extra time/device/safety)
Location: Toilet
Assistive Device/Equipment: Walker
Pt. Will Perform Transfers: Yes
Level of Assistance: Modified Independent (extra time/device/safety)
Location: Toilet
Assistive Device/Equipment: Walker

[table#4]:
Plan

Continue with Plan of Care,

[table#5]:
PT/OT/SLP Recommendations

[table#6]:
Flowsheet Row	Most Recent Value
PT Dispo Recommendations	SNF filed at 01/23/2024 0925
PT Assistive Device Needs	FWW filed at 01/23/2024 0925
PT Safety Concerns	falls risk, limited social support, limited ac
tivity and positional tolerance filed at 01/23/2024 0925
OT Dispo Recommendations	SNF (P) filed at 01/23/2024 1549
OT Safety Concerns	fall risk, homeless (P) filed at 01/23/2024 1549
SLP Safety Concerns	Aspiration Precautions filed at 01/15/2024 1022
SLP Discharge Summary	Dysphagia 3/all liquids filed at 01/15/2024 1022
SLP Discharge Dispo Recommendation	Home filed at 01/15/2024 1022

[table#7]:

Thank you for including Inpatient Rehabilitation in the care of Elizabeth Tina Petruccio.

Elaheh Sobhani, OT
1/23/2024

Electronically signed by Elaheh S at 01/23/2024 4:09 PM PST

PT Treatment Note - Deborah S - 01/23/2024 9:25 AM PST
Formatting of this note might be different from the original.
Physical Therapy
[table#1]:
| ***Physical Therapy - Treatment Note***
***Scripps Mercy Hospital *** |
| --- |

[image]

Patient Name: Elizabeth Tina Petruccio

Admission Date: 1/12/2024
Eval Date: 1/23/2024
Code Status: Full Code
Unit/Room: 0426/01

Social History
Type of Residence: Shelter
Prior Level of Function Mobility : Independent, Modified Independent (extra time/device/safety)
Assistive Device : (none)
Living Arrangements: Alone
Communication: WNL
Prior Level of Function
Functional Mobility: Independent
ADLs: Independent

[table#2]:
SUBJECTIVE

Primary Diagnosis: The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

[table#3]:
OBJECTIVE

Relevant Lines/Drains:
[table#4]:
Patient Devices

[table#5]:
| | [table#5.1]
[table#5.2]
[table#5.3] |
| --- | --- |

[table#5.1]:
Active Devices

[table#5.2]:
	Name	Placement date	Placement time	Site	Days
	Midline Single Lumen 01/14/24 Right Basilic	01/14/24	0215	Basilic	9
	External Urinary Device	—	—	—	10

[table#5.3]:

[table#6]:

[table#7]:
	01/23/24 0925
PT Treatment
PT Treat Start Time	0925
PT Treat End Time	0950
PT Treatment Total minutes	25 minutes
**Subjective **
Subjective	Agreeable, states her pain is well managed.
Interpreter Services
Communication Needs	None
Precautions/Limitations
Precautions	Fall risk
Pain Assessment

| Pain Assessment | No/denies pain |
| **Balance** |
Sitting Static	Good - w/UE
Sitting Dynamic	Good - w/UE
Standing Static	Fair w/AD
Standing Dynamic	Fair - w/AD
Bed Mobility Training
Bed Mobility (Y)	Yes
Rolling R	Modified Independent (extra time/device/safety)
Rolling L	Modified Independent (extra time/device/safety)
Sidelying to SIt	Modified Independent (extra time/device/safety)
Sit to Sidelying	Modified Independent (extra time/device/safety)
Supine to Sit	Modified Independent (extra time/device/safety)
Sit to Supine	Modified Independent (extra time/device/safety)
Transfers
Transfer (Y)	Yes
Sit to Stand	Contact Guard Assist
Stand to Sit	Contact Guard Assist
Bed to Chair	Contact Guard Assist
Chair to Bed	Contact Guard Assist
Gait Training
Gait Assistance	Contact Guard Assist
Gait Distance (ft)	50 ft
Gait Assistive Device	FWW
Safety Awareness
Safety Awareness with Functional Mobility	Fair
Neuro Assessment
Alertness	Intact
Orientation Level	Oriented X4
PT Discharge Recommendations
PT Discharge Dispo Recommendations	SNF
PT Discharge Assistive Device Needs	FWW
PT Safety Concerns	falls risk, limited social support, limited activity and positional tolerance

[table#8]:
ASSESSMENT

**Assessment: ** Pre and post huddle with RN Gladdyz D. Pt greeted in supine and agreeable to PT. Mobility per flow sheet. Pt BTB with all needs, (+) bed alarm. Appropriate progress towards goals. Pain well managed within current session. Pt seems fatigued post mobility as she seems to fall asleep promptly upon returning BTB. Continue with PT POC. D/c SNF indicated.

**Functional Goals: **Short Term Goals
Short Term Goals Due By: 02/04/24
Short Term Goal (Y): Yes
Pt. Will Perform Bed Mobility: Independent
Pt. Will Perform Transfer : Modified Independent (extra time/device/safety) (LRAD)
Pt. Will Perform Gait: Modified Independent (extra time/device/safety) (LRAD, 100 feet)
Pt. Will Go Up / Down Stairs: Modified Independent (extra time/device/safety((LRAD, curb management for community access)

**Recommendations to Nursing: **Ambulate multiple times daily with nursing assistance, utilize FWW

[table#9]:
PLAN

Plan: Continue Physical Therapy as tolerated and appropriate per plan of care. ** **

Thank you for including Inpatient Rehabilitation in the care of Elizabeth.

Deborah A Schweer, PT
1/23/2024

Electronically signed by Deborah S at 01/23/2024 9:54 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Received pt A&Ox4, NAD, calm and cooperative with plan of care. PRN dilaudid and oxycodone administered per order for moderate to severe right ankle pain management, effective per pt. PO intake well tolerated, denies N/V, 1.5L fluid restriction monitored. Pt is on 2lpm via NC, mild SOB w/ exertion noted, SPO2 at >90%. Fall and safety precautions in place. Explained plan of care, meds, and tx, and provided instructions on the use of call button for assistance, pt verbalized understanding and agreement. Continuing plan of care.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: none

1211 - Pt had 14beats of Vtach. Now NSR on tele, remains A&Ox4, denies pain or discomfort at this time, all vitals WNL. Dr. Khayyat was informed.

Electronically signed by Your Scripps Health Team at 01/23/2024 9:20 AM PST
Electronically signed by Your Scripps Health Team at 01/23/2024 12:24 PM PST

Plan of Care - Your Scripps Health Team - 01/23/2024 3:54 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt continues with plan of care. Pt is A&O x3, disoriented to date, anxious but cooperative with all nursing interventions. Pt has been c/o around the clock severe to moderate RLE ankle pain; PRN IV Dilaudid was given 2x, PRN Naproxen and Oxycodone were also given with mild to moderate pain relief. Around midnight, pt requested a topical solution to her unrelieved RLE ankle pain; MD Y. Piatetsky was notified, ordered a Lidocaine patch, and medication was applied with mild pain relief. Pt's RLE ankle remains elevated, and heat / cold packs are being applied intermittently. Pt started on PO antibiotics; one dose of Augmentin was given during my shift. Pt's RL flank chest tube excision dressing remains clean, dry, and intact with no signs of drainage. Pt midline IV access remains patient, dressing is clean, dry, and intact, and all connections have been checked and tightened. Pt's telemetry strip revealed NSR with a QTC in the yellow zone; pharmacy is aware. Pt has been oriented to the unit, and displays appropriate safety awareness and all safety measures are in place; bed in the lowest position, bedside table within reach, bed alarm on, and call light at arm's length. Pt's VSS, denies chest pain, or any discomfort at this time, and is not in acute distress. Pt is currently resting in bed, comfortably.

**Key findings related to barriers to transfer/discharge: **SNF placement. Medical clearance.

Patient/Family needs and concerns: Pain management.

Electronically signed by Your Scripps Health Team at 01/23/2024 4:05 AM PST

Plan of Care - Your Scripps Health Team - 01/22/2024 6:57 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Appropriate on approach; had episode of faint expiratory and inspiratory wheezing; eached out to Francisco, RCP for neb tx and administered; no desaturation episode noted; remains on O2 at 2lpm via NC with O2 sat within parameter; r; got up to BSC for BM and went successfully; sat on EOB; RD came by and followed up with patient; diet updated; still with ongoing FR of 1500ml; dental soft combination regular; purewick device use; encourage importance of good skin integrity, fall prevention and safety precautions; rt ankle puncture site and rt flank CT site appears C/D/I; current POC ongoing; medicated with Dialudid 0.5mg IVP for right side pain- verbalized relief

**Key findings related to barriers to transfer/discharge: **belongings reconciled; reached out to security, 312-7465, s/w Donald to coordinate above request.

Patient/Family needs and concerns: Sore throat; cough- addressed and medicated as ordered.

1110hrs:
Hosppital tablet provided to patient

1134hrs:
Valuables Envelope number 14596939; coordinated with Donald and Darrell- Security staff; copy of valuable receipt provided to patient.

Problem: PAIN - ADULT
Goal: Patient/Family/Caregiver verbalizes satisfaction with comfort level
Outcome: Progressing

Problem: INFECTION - ADULT
Goal: Absence of infection during hospitalization
Outcome: Progressing

Problem: FALL PREVENTION
Goal: No falls during hospitalization
Outcome: Progressing

Problem: SKIN BREAKDOWN PREVENTION
Goal: Skin Integrity is Maintained or Improved
Outcome: Progressing

Problem: DISCHARGE PLANNING
Goal: Discharge to home or other facility with appropriate resources
Outcome: Progressing

Problem: Inadequate Gas Exchange
Goal: Patient is adequately oxygenated and ventilation is improved
Outcome: Progressing

Electronically signed by Your Scripps Health Team at 01/22/2024 6:57 PM PST

Plan of Care - Your Scripps Health Team - 01/21/2024 10:09 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Oriented x3 but forgetful and has trouble focusing on one thing at a time. Saturates 90-93 on room air when removes NC, up to 100% on 1-2L NC, no SOB. SR or ST to 110. Compliant with fluid restriction. Dr Que performed aspiration of R ankle, obtained <1 mL bloody fluid, per lab insufficient quantity to perform any testing.

**Key findings related to barriers to transfer/discharge: **DC planning

Patient/Family needs and concerns: None

Electronically signed by Your Scripps Health Team at 01/21/2024 10:12 PM PST

Plan of Care - Your Scripps Health Team - 01/21/2024 3:33 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt lying in bed, allowed to assume comfortable position. With even and unlabored breathing. Does not appear in distress. Given PRN pain meds for neck pain and abdominal pain. Due meds given and tolerated well. Around 1500 pt complains of feeling hot, dizzy and sweaty. O2 sats reading in high 90's on 2L oxygen. Vital signs stable, temp 37.3. Given PRN hydroxyzine and encouraged to try and get some rest. Encouraged to call for assistance if needed.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: No concerns reported at this time. Request for PRN medications communicated with MD, orders in place for anxiety and acid reflux.

1648 RN Aura performing midline dressing change, noted ecchymosis around biopatch and under midline catheter/tubing. Photo taken and notified Dr. Chen via secure chat. Denies any pain around site.

Electronically signed by Your Scripps Health Team at 01/21/2024 3:36 PM PST
Electronically signed by Your Scripps Health Team at 01/21/2024 6:02 PM PST
Electronically signed by Your Scripps Health Team at 01/21/2024 6:02 PM PST

PT Evaluation - Deborah S - 01/21/2024 8:31 AM PST
Formatting of this note might be different from the original.
Physical Therapy
[table#1]:
| ***Physical Therapy - Evaluation ***
***Scripps Mercy Hospital *** |
| --- |

[image]

Patient Name: Elizabeth Tina Petruccio
Admission Date: 1/12/2024
Eval Date: 1/21/2024
Code Status: Full Code
Unit/Room: 0426/01

[table#2]:
SUBJECTIVE

History of Present Illness/Diagnosis: Elizabeth Tina Petruccio is a 59 y.o. female who presents to Mercy Hospital with RF, COPD, pleural effusion, oral thrush, AKI, prolonged QT interval, thoracecntesis, intubated 1/13-1/14, R empyema, CHF, afib w RVR, asthma, chest tube 1/13-1/21, transaminitis, R ankle pain

[table#3]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart
failure) (CMS/HCC)	

Past Surgical Intervention: Elizabeth has no past surgical history on file.
Pertinent Social History: Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

[table#4]:
OBJECTIVE

Relevant Lines/Drains:
[table#5]:
Patient Devices

[table#6]:
| | [table#6.1]
[table#6.2]
[table#6.3] |

| --- | --- |

[table#6.1]:
Active Devices

[table#6.2]:
	Name	Placement date	Placement time	Site	Days
	Midline Single Lumen 01/14/24 Right Basilic	01/14/24	0215	Basilic	7
	External Urinary Device	—	—	—	8

[table#6.3]:

[table#7]:

[table#8]:
	01/21/24 0831
Assessment Type
Assessment Type	Initial
Chart Reviewed	Yes
Referring MD	Dao, Diep Ngoc MD
Medical Diagnosis	RF, COPD, pleural effusion, oral thrush, AKI, prolonged QT interval, thoracecntesis, intubated 1/13-1/14, R empyema, CHF, afib w RVR, asthma, chest tube 1/13-1/21, transaminitis, R ankle pain
PT Evaluation Start Time	0831
PT Evaluation End Time	0901
PT Evaluation Total minutes	30 minutes
**Subjective **
Subjective	Agreeable. Hyperverbal, tangential.
Social History
Type of Residence	Shelter
Prior Level of Function Mobility	Independent;Modified Independent (extra time/device/safety)
Living Arrangements	Alone
Communication	WNL
Interpreter Services
Communication Needs	None
Pain Assessment
Pain Assessment	0-10
Pain Score	7
Pain Location
(B ankle/ feet)
Pain Interventions	Repositioned;RN Notified
Vitals/Summary
Time of Reading	During treatment
Patient Position	Sitting
BP	101/72
Heart Rate	103
SpO2	91 %
Oxygen Therapy	Supplemental oxygen
O2 Flow Rate (l/min)	1
Precautions/Limitations
Precautions	Aspiration;Fall risk
Functional Assessment
Rolling R	Modified Independent (extra time/device/safety)
Rolling L	Modified Independent (extra time/device/safety)
Scooting	Modified Independent (extra time/device/safety)
Sidelying to SIt	Supervision
Sit to Sidelying	Supervision
Supine to Sit	Supervision
Sit to Supine	Stand by Assist
Sit to Stand	Contact Guard Assist
Stand to Sit	Contact Guard Assist
Gait Assistance	Minimum Assist (Subject 75% or more)
Gait Distance (ft)	10 ft
Gait Assistive Device	FWW

| **Safety Awareness** |
| Safety Awareness with Functional Mobility | Fair |
| **Neuro Assessment** |
Alertness	Intact
Orientation Level	Oriented X4
Coordination	Impaired
Balance
Sitting Static	Fair +
Sitting Dynamic	Fair +
Standing Static	Poor + w/AD
Standing Dynamic	Poor + w/AD
Range of Motion
Gross LLE	WFL
Gross RLE	WFL
Muscle Strength
Gross LLE	Impaired
Gross RLE	Impaired
Skin Care Assesment
Skin
(Per RN Jia)
Interpretation of Findings
Functional Mobility Limitations	Bed mobility;Transfer;Gait velocity;Gait with/without AD;Stairs
Impairments	Activity tolerance;Balance;Coordination;Limited knowledge of impairment;Muscle Strength;Pain;Posture;Respiratory capacity;Safety awareness;Other
Rehab Potential	Fair
Age-Specific Considerations	Adult
Plan of Care
Plan of Care Frequency/Duration	5 times/week;2 weeks
Therapy Interventions	Bed Mobility Training;Transfer Training;Gait Training;Stair Training;Therapeutic Exercises;Home Exercise Program;Neuromuscular Re-education;Body Mechanics;Balance activities;Patient/ Caregiver Education
Patient's Goal
Patient's Stated Goal	none stated
Risks/benefits/goals of therapy discussed with patient/caregiver	Yes
Pt./family understand and concur w/purpose & goals	Yes
Short Term Goals
Short Term Goals Due By	02/04/24
Short Term Goal (Y)	Yes
Pt. Will Perform Bed Mobility	Independent
Pt. Will Perform Transfer	Modified Independent (extra time/device/safety) *(LRAD)*
Pt. Will Perform Gait	Modified Independent (extra time/device/safety) *(LRAD, 100 feet)*
Pt. Will Go Up / Down Stairs	Modified Independent (extra time/device/safety(*(LRAD, curb management for community access)*
6 Clicks Basic Mobility PT
Turning from your back to your side while in a flat bed without using bedrails?	3
Moving from lying on your back to sitting on the side of a flat bed without using bedrails?	3
Moving to and from a bed to a chair (including a wheelchair)?	3
Standing up from a chair using your arms (e.g., wheelchair or bedside chair)?	3
Walking in hospital room?	1
Climbing 3-5 steps with a railing?	1
PT 6 Clicks Score	14
PT Discharge Recommendations
PT Discharge Dispo Recommendations	SNF
PT Discharge Assistive Device Needs	FWW
PT Safety Concerns	falls risk, pain limiting function, limited social support, limited activity and positional tolerance

[table#9]:
ASSESSMENT

**Assessment: ** Pre and post huddle with RN Jia. Pt greeted in supine and agreeable to PT. Mobility per flow sheet. Pt BTB with all needs, (+) bed alarm. Pt currently performs at functional deficit compared to reported PLOF and would benefit from continued PT to improve safety and independence with mobility prior to d/c SNF. FWW utilized/ indicated currently.

**Recommendations to Nursing: **Transfer to, and sit in chair multiple times daily, utilize FWW

[table#10]:
| ***PLAN*** |

| --- |

Plan: Elizabeth will benefit from continued Physical Therapy treatment while admitted in house to address and treat her identified mobility deficits and to facilitate return to her prior level of function as well as to optimize safe mobility while admitted.

Thank you for including Inpatient Rehabilitation in the care of Elizabeth.

Deborah A Schweer, PT
1/21/2024

Electronically signed by Deborah S at 01/21/2024 9:27 AM PST

Plan of Care - Your Scripps Health Team - 01/21/2024 4:42 AM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: A+OX3. Afebrile, denies SOB, nausea, vomit or diarrhea. With NC at 2L. ST on telemetry. With chest tube secured in place. Patient refused Flagyl at bedtime stating she's allergic to it for many years (although patient had one dose at 11:00 and she didn't have any allergic reaction). Dr Dao, was notified. Medication PRN for pain given with good effect. All safety precautions in place. Will continue with plan of care.

**Key findings related to barriers to transfer/discharge: **medical clearance

Patient/Family needs and concerns: met

Electronically signed by Your Scripps Health Team at 01/21/2024 4:49 AM PST

Plan of Care - Your Scripps Health Team - 01/20/2024 12:56 PM PST

Formatting of this note might be different from the original.

Nursing Summary of Patient Progress

Important trends/observation and significant events: Patient resting on the bed, alert, oriented x3 on oxygen at 2 liters nasal cannula, desaturates on exertion. Chest tube on the right lateral side of the chest, connected to suction, draining serosanguinous drainage. Seen by OT today, desaturates even on oxygen on exertion. Patient stood up but had a lot of pain on the right foot. Dr Dao saw patient, will increase the dose for colchicine per MD. Updated the allergy list and added lactose intolerance as reported by patient.

**Key findings related to barriers to transfer/discharge: **

Patient/Family needs and concerns:

Electronically signed by Your Scripps Health Team at 01/20/2024 1:22 PM PST

OT Evaluation - Elaheh S - 01/20/2024 9:25 AM PST

Formatting of this note might be different from the original.

** Occupational Therapy**

[image]

Patient Name: Elizabeth Tina Petruccio
Admission Date: 1/12/2024
Eval Date: 1/20/2024
**U
nit/Room:** 0407/01
Code Status: Full Code

**Primary Diagnosis: **The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

History of Present Illness: Elizabeth Tina Petruccio is a 59 y.o. female who presents to Scripps Mercy Chula Vista with

[table#1]:
Chief Complaint
Patient presents with
•	Shortness of Breath
		Pt. Is a 59 y.o. female BIBM from border patrol CC: Shortness of breath onset today. Pt. Has history of COPD, CHF, AFIB

Past Medical History:
[table#2]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

Past Surgical Intervention: Elizabeth has no past surgical history on file.
Pertinent Social History: Elizabeth reports that she has quit smoking. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs.

[table#3]:
Subjective / Objective / Assessment

[table#4]:
	01/20/24 0925
Assessment Type
Assessment Type	Initial
Chart Reviewed	Yes
Referring MD	Ani, Samuel, MD
Medical Diagnosis	Acute respiratory failure with hypercapnia
OT Evaluation Start Time	0925
OT Evaluation End Time	0940
OT Evaluation Total minutes	15 minutes
OT Treatment Start Time	0940
OT Treatment End Time	0955
OT Treatment Total Minutes	15 minutes
Additional Comments	15 min ADLs
**Subjective **
Subjective	RN cleared pt for OT; pt agreeable.
Social History
Type of Residence	Homeless
Assistive Device
(none)
Living Arrangements	Alone
Interpreter Services
Communication Needs	None
Prior Level of Function
Functional Mobility	Independent
ADLs	Independent
Precautions/Limitations
Precautions	Aspiration;Fall risk
Pain
Pain Assessment	Verbal Descriptor Scale (VDS)
VDS Pain Score	Severe Pain (Pain Level 7-10)
Pain Type	Acute pain
Pain Location	Abdomen
(at chest tube site; and pain in R foot)
Pain Orientation	Right
Pain Descriptors	Aching
Pain Frequency	Constant/continuous
Pain Aggravating Factors	Reposition/movement;Other (Comment);Standing;Walking
(sitting)
Patient's Stated Pain Goal	No pain
Pain Interventions	Medication (See MAR)
Response to Interventions	effective
Vitals/Summary
Time of Reading	During treatment
Patient Position	Ambulating
SpO2	90 %
Oxygen Therapy	Supplemental oxygen
O2 Flow Rate (l/min)	2
Functional Assessment
Supine to Sit	Moderate Assistance (Subject 50-74%)
Sit to Supine
(pt remained seated at EOB)
Sit to Stand	Moderate Assistance (Subject 50-74%)

(FWW) |
| Stand to Sit | Minimal Assistance (Subject 75% or more) |
| Commode/Toilet Transfer | Minimal Assistance (Subject 75% or more)
(BSC) |
| **Occupational Function** |
ADL/IADL (Y)	ADL
Self Feeding	Independent
Hygiene/Grooming	Set-up
Dressing UE	Moderate Assistance (Subject 50-74%)
Dressing LE	Maximal Assistance (Subject 25-49%)
Toileting/Pericare	Moderate Assistance (Subject 50-74%)
Safety Awareness
Safety awareness with ADL/IADL	Good
Safety awareness with functional mobility	Good
Balance
Sitting Static	Good - w/UE
Sitting Dynamic	Fair + w/UE
Standing Static	Fair w/AD
Standing Dynamic	Fair - w/AD
Range of Motion
Gross LUE	WFL
Gr
oss RUE	WFL
**Muscle Strength **
Gross LUE	WFL
Gross RUE	WFL
Skin Care Assesment
Skin	Impaired
(R side ches tube)
Cognition
Arousal/Alertness	Appropriate responses to stimuli
Orientation Level	Oriented to person;Oriented to place
Judgement	Good
Hearing
Hearing	Intact
Vision
Current Vision	Does not wear glasses
Interpretation of Findings
ADL Limitations	Hygiene;Dressing;Bathing;Toileting
Impairments	Activity tolerance;Balance;Muscle Strength;Pain;ROM;Respiratory capacity
Rehab Potential	Good for Stated Goals
Age-Specific Considerations	Adult
Assessment
Assessment	Pt pleasant and cooperative. She presents below baseline for ADLs and functional mobility d/t gen. weakness, decreased balance, decreased activity tolerance, pain in chest tube site and R foot, and decreased flexibility. Pt would benefit from skilled OT service to maximise safety and IND w/ ADLs and functional mobility.
Plan of Care
Plan of Care Frequency/Duration	1 time/day;5 times/week;2 weeks
Therapy Interventions	ADL Retraining;Adaptive Techniques;Adaptive Equipment Training;Balance;Energy Conservation Techniques;Fall Prevention;Patient/Family Education;Strengthening;Therapeutic Exercise
Patient's Goal
Patient's Stated Goal	to get better
Risks/benefits/goals of therapy discussed with patient/caregiver	Yes
Pt./family understand and concur w/purpose & goals	Yes
Short Term Goals
Short Term Goals Due By	02/03/24
Pt. Will Perform LE Dressing	Yes
Level of Assist	Modified Independent (extra time/device/safety)
Location	EOB
Assistive Device/Equipment	Walker
Pt. Will Perform Toileting	Yes
Level of Assistance	Modified Independent (extra time/device/safety)
Location	Toilet
Assistive Device/Equipment	Walker
Pt. Will Perform Transfers	Yes
Level of Assistance	Modified Independent (extra time/device/safety)
Location	Toilet
Assistive Device/Equipment	Walker
Treatment Note
Treatment Note	Assisted pt w/ toilet transfer and toileting task over bedside commode w/ education on proper foot/hand

placement during transfers using FWW. |
| **6 Clicks Daily Activity OT** |
Putting on and taking off regular lower body clothing?	2
Bathing (including washing, rinsing, drying?)	2
Toileting, which includes using toilet, bedpan, or urinal?	2
Putting on and taking off regular upper body clothing?	2
Taking care of personal grooming such as brushing teeth?	3
Eating meals?	4
OT 6 Clicks Score	15
OT Discharge Recommendations
OT Discharge Dispo Recommendations	SNF
OT Safety Concerns	fall risk; homeless

Patient returned/left seated at EOB w/ R foot elevated with:
[x] All needs within reach
[x] RN updated on patient's status.
[x] Other: bed alarm on

Education Provided:
- **Learner: ** [x]Patient** [ ]**Caregiver **[ ]**Family **[ ]** Other:
- **Barriers to Learning:** [x]None [ ]Language [ ]Literacy [ ]Vision [ ]Memory [ ] Hearing [ ]Other:
- **Learning Style:** [x]Verbal [ ]Written [x] Demonstration [ ] Other:

[table#5]:
	***NFT**	**PS**	**VU**	**NVA**	**RD**	**RE**
Role of Occupational Therapy	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Using Nursing Call Light	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Fall Precautions	[ ]	[ ]	[x]	[ ]	[ ]	[ ]
Adaptive Equipment / Techniques	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Home Exercise Program	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Positioning/Mobility	[ ]	[x]	[ ]	[ ]	[ ]	[ ]
Therapeutic Exercise	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
Other:** **	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
***NFT** = needs furthe
r teaching** , PS**= requires practice/supervision, **VU**=verbalized understanding,
NVA=non-verbal acknowledgement; **RD**=return demonstration, **RE**=Refused Education

Please see future OT notes for updates for discharge recommendations and equipment needed.

[table#6]:
Plan

Occupational Therapy treatment as tolerated until functional goals met or patient is discharged from facility.

Thank you,
Elaheh Sobhani, OTR/L

Electronically signed by Elaheh S at 01/20/2024 11:49 AM PST

Plan of Care - Your Scripps Health Team - 01/19/2024 8:48 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: A/O x 3; reoriented to time; ST on the monitor; denies chest pains; on 2L O2 via NC saturation 97 %; w/ SOB on exertion; w/ chest tube to 20 cm water continuous suction. Agnes RN CCO up here in the unit to assist this primary RN for change of chest tube container; Oasis chest tube container connected to the pt and to 20 cm water to continuous wall suction; procedure tolerated well; pt not in any form of distress; chest tube container working well. Repositioned pt for comfort; fall precautions observed.

1/20 @ 0005- awoken very anxious; SOB;Dr Jose Pena aware and with order; BP 113/89; RR 24; O2 sat on 2 L 95%; Ativan 1 mg IVP given as ordered.

0025- Pt c/o of SOB and difficulty breathing; requesting for breathing treatment; Dr J. Pena aware; w/ orders and carried out. Kiel,RT aware pt needs breathing treatment; awaiting for PCXR to be done. PCXR done and waiting for results.

0100- Pt more calm now; not in respiratory distress;O2 sat on 2L via NC 98%. Still waiting for PCXR result.

0400- pt resting comfortably; denies any discomfort; O2 sat on 2L via NC 97%; Jasmine, XRay tech notified no results yet of PCXR done; per info will notify STAT Rad.

Key findings related to barriers to transfer/discharge: Medical clearance

Patient/Family needs and concerns: None

Electronically signed by Your Scripps Health Team at 01/19/2024 8:56 PM PST
Electronically signed by Your Scripps Health Team at 01/20/2024 12:42 AM PST
Electronically signed by Your Scripps Health Team at 01/20/2024 3:14 AM PST
Electronically signed by Your Scripps Health Team at 01/20/2024 3:52 AM PST
Electronically signed by Your Scripps Health Team at 01/20/2024 4:09 AM PST

Plan of Care - Your Scripps Health Team - 01/19/2024 1:44 PM PST
Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: pt A&Ox4, c/o migraine and pain to R back (chest tube site), PRNs given. On 2L NC, no resp distress. ST w/ PVCs.

1345: chest tube unclamp @ 1245, noticed serosanguinous drainage drain in the chest tube. Just now went to recheck on the pt, noticed serosanguinous drainage soaking the chest tube dressing and small to moderate amount on the bed sheet. Pt sating 99% on 2L. Dr. Dammert notified.

1400: pt c/o severe migraine, Dr. Ani notified. Per MD, apply cold compress and give naproxen. If still having migraine, repeat cold compress.

1410: Dr. Dammert @ bedside, 20ml NS flushed thro chest tube by MD. One time IV Dilaudid given per order. Bedside CXR done. DSG to chest tube changed.

Key findings related to barriers to transfer/discharge: medical clearance

Patient/Family needs and concerns: N/A

Electronically signed by Your Scripps Health Team at 01/19/2024 1:45 PM PST
Electronically signed by Your Scripps Health Team at 01/19/2024 3:03 PM PST

PT Patient Not Seen - Gordon I - 01/19/2024 11:11 AM PST

Formatting of this note might be different from the original.
Physical Therapy
[image]

Physical Therapy Deferral Note

Patient Name: Elizabeth Tina Petruccio
Unit/Room: 0407/01
**Primary Diagnosis: **The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

**Date: **1/19/2024

**Reason For Deferral: **
[ ] Patient Refused
[ ] Family Refused
[ ] Therapist Deferred
[x] RN Deferred
[ ] MD Deferred
[ ] Medical Contraindication
[ ] Awaiting Consultation
[ ] Pt Unable to Participate
[ ] Pt Unavailable for Treatment
[ ] Scheduling Conflict
[ ] Patient At or Pending Surgery
[ ] Other:

**Comment: **Per RN, pt had migraine in morning. Upon arrival at room RN further emphasized deferral today.

Thank you,
Gordon Ip, PT, DPT

Electronically signed by Gordon I at 01/19/2024 12:45 PM PST

Plan of Care - Your Scripps Health Team - 01/18/2024 11:49 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events:

Pt AAOX3, at times forgetful, calm and cooperative with nursin
g care. On 1L NC with expiratory wheezes, RT notified for PRN duoneb. Remains with R chest tube in place to continuous suction. Reports improvement in pain management with oral lesions/sores. NSR on tele monitoring. Fall and safety precautions in place, call light within reach, educated pt on use of call light for assistance.

**Key findings related to barriers to transfer/discharge: **

Medical clearance.

Patient/Family needs and concerns:

None identified at this time.

Electronically signed by Your Scripps Health Team at 01/18/2024 11:54 PM PST

Plan of Care - Nurse Sue K - 01/18/2024 6:31 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: A&Ox4. Cooperative with care but anxious at times. VSS on 1L NC. R chest tube no output until it was flushed by Dr. Dammert; 90ml yellow cloudy output during this shift. Duoneb x1 for SOB which was effective. Tolerating diet and bed repositioning. Pt reports severe R foot/ankle pain; MD notified. PRN naproxen and oxycodone given for pain. Mouth with herpes lesions red and excoriated. Had 2 loose Bms.

Key findings related to barriers to transfer/discharge: Pleural effusion requiring chest tube

Patient/Family needs and concerns: requesting to talk to SW tomorrow morning

Electronically signed by Nurse Sue K at 01/18/2024 6:32 PM PST

PT Patient Not Seen - Gordon I - 01/18/2024 1:41 PM PST

Formatting of this note might be different from the original.
Physical Therapy
[image]

Physical Therapy Deferral Note

Patient Name: Elizabeth Tina Petruccio
Unit/Room: 0407/01
**Primary Diagnosis: **The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

**Date: **1/18/2024

**Reason For Deferral: **
[ ] Patient Refused
[ ] Family Refused
[ ] Therapist Deferred
[ ] RN Deferred
[ ] MD Deferred
[ ] Medical Contraindication
[ ] Awaiting Consultation
[ ] Pt Unable to Participate
[x] Pt Unavailable for Treatment
[x] Scheduling Conflict
[ ] Patient At or Pending Surgery
[ ] Other:

**Comment: **Pt in radiology and unavailable for therapy at this time.

Thank you,
Gordon Ip, PT, DPT

Electronically signed by Gordon I at 01/18/2024 4:48 PM PST

Plan of Care - Your Scripps Health Team - 01/18/2024 2:23 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events:

Pt AAOX4, with complaints of severe pain to sores on mouth, PRN pain medication, lidocaine and scheduled meds administered with effect in pain relief. Pt on 1L NC with R chest tube in place, purulent drainage, site with redness and tender to touch. Remains on NS infusing continuous @ 50mL/hr. IV ABX administered. Fall and safety precautions in place, call light within reach, educated pt on use of call light for assistance.

**Key findings related to barriers to transfer/discharge: **

Medical clearance.

Patient/Family needs and concerns:

None identified at this time.

Electronically signed by Your Scripps Health Team at 01/18/2024 2:25 AM PST

Plan of Care - Your Scripps Health Team - 01/17/2024 12:00 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt is anxious, AO X4, follows commands, pt has episodes of A-fib/A-flutter with HR 110-150s and NSR/ ST with HR 90-100s at rest, with right chest tube at continuous suction with large yellow/ purulent output, erythema, pain and swelling at insertion site, MD made aware, with SOB at rest, on exertion or while laying flat, pulmonologist checked chest tube, flushed it at bedside, pt denies and chest pain, pt is on Acyclovir for mouth sores, and on IV antibiotics. Continue to monitor pt, labs and VS.

Key findings related to barriers to transfer/discharge: Medical clearance

Patient/Family needs and concerns: none at this time

Electronically signed by Your Scripps Health Team at 01/17/2024 1:59 PM PST

Plan of Care - Your Scripps Health Team - 01/17/2024 5:58 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Plan of care revolving. AAO X3, somewhat anxious. Follows commands, ST with HR 100-140s with movement. Rt sided chest tube at continuous suction with moderate purulent output. Intermittent SOB and generalized pain. Call light within re
ach and frequent rounding. Prn respiratory medication and pain medication. Will continue to monitor closely.
0635: Informed by tele that pt had 25 second runs of A-Fib/A- Flutter 110 up to 149 non-sustained. Asymptomatic, pt asleep. VSS. Resident notified with no new orders.

**Key findings related to barriers to transfer/discharge: **Medical clearance

Patient/Family needs and concerns:

Electronically signed by Your Scripps Health Team at 01/17/2024 6:09 AM PST
Electronically signed by Your Scripps Health Team at 01/17/2024 7:24 AM PST

Plan of Care - Your Scripps Health Team - 01/16/2024 1:35 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt is anxious, AO X4, follows commands, irritable at times, pt is ST with HR 100-110s at rest, wit hright chest tube at continuous suction with yellow/purulent output, with SOB and tachypneic at rest, on exertion and while laying flat, pulmonologist checked chest tube, flushed it at bedside and ordered CT chest STAT in AM, pt denies and chest pain, she complains of generalized pain, prn Dilaudid helps, pt is on Acyclovir IV for mouth herpes/sores, and on IV antibiotics. Continue to monitor pt, labs and VS.

Key findings related to barriers to transfer/discharge: Pending TPA/DNAs at bedside today by MD

Patient/Family needs and concerns: pt's son is at bedside

Electronically signed by Your Scripps Health Team at 01/16/2024 1:40 PM PST
Electronically signed by Your Scripps Health Team at 01/16/2024 1:41 PM PST

Plan of Care - Your Scripps Health Team - 01/16/2024 5:51 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt AOx4, ST on the monitor now, some rapid aflutter to 150s overnight, temp 38.1 at midnight vitals but resolved. BP soft. Frequent complaints of pain, screaming and moaning loudly, r/t sores in mouth/ throat with PRNs used. CT right side to 20cm suction with minimal output overnight.

2309 aflutter 150s
2326 metoprolol 5mg IV x1
2340 NSR 91

**Key findings related to barriers to transfer/discharge: **Medical clearance.

Patient/Family needs and concerns: Pain control for sores in mouth/throat.

Electronically signed by Your Scripps Health Team at 01/16/2024 5:59 AM PST

PT Patient Not Seen - Gordon I - 01/15/2024 12:58 PM PST

Formatting of this note might be different from the original.
Physical Therapy
[image]

Physical Therapy Deferral Note

Patient Name: Elizabeth Tina Petruccio
Unit/Room: 0412/02
**Primary Diagnosis: **The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

**Date: **1/15/2024

**Reason For Deferral: **
[ ] Patient Refused
[ ] Family Refused
[ ] Therapist Deferred
[ ] RN Deferred
[ ] MD Deferred
[ ] Medical Contraindication
[ ] Awaiting Consultation
[ ] Pt Unable to Participate
[x] Pt Unavailable for Treatment
[ ] Scheduling Conflict
[ ] Patient At or Pending Surgery
[ ] Other:

**Comment: **Pt in radiology. Will reattempt later today if schedule permits.

Thank you,
Gordon Ip, PT, DPT

Electronically signed by Gordon I at 01/15/2024 1:01 PM PST

SLP Evaluation - Rosemarie S - 01/15/2024 10:22 AM PST
Formatting of this note might be different from the original.
[table#1]:
| **SPEECH LANGUAGE PATHOLOGY**
**Inpatient Clinical/Bedside Swallow Evaluation **
Scripps Mercy Hospital Chula Vista |
| --- |

Date: 1/15/2024
Patient: Elizabeth Tina Petruccio
MRN: 810760324
Referring Physician: Strauser, Martha Dalia, MD
Date of Admission: 1/12/2024
Admission Diagnosis: Oral thrush [B37.0]
Pleural effusion [J90]
Prolonged QT interval [R94.31]
COPD exacerbation (CMS/HCC) [J44.1]
Acute kidney injury (CMS/HCC) [N17.9]
Acute respiratory failure with hypercapnia (CMS/HCC) [J96.02]
LOS: 3

Pain Assessment (current): 10/10
Pain Behavior: Constant
Pain Location: inside of mouth, namely cheeks and tongue
Notified Nurse: Yes - RN stated that she was already aware of pt's pain

Patient behavior: Alert, Cooperative, and Impulsive

Time In/Time Out: 1022-1037
Total Minutes: 15
Care team notified of ST recommendations (including any changes to previous recommendations) at: 1038 via Secure Chat

Therapist PPE Use: N95 Mask and Eye Shield SLP temporary mask doffing for essential therapeutic purposes: No, length of time: n/a

Reason for Referral: Assess swallow function

History
Current and past history were reviewed and information was obtained from: Patient and Medical Record

**H&P: **59 y.o. female with past medical history of heart failure reduced ejection fraction on echo, reported history of asthma unclear if she had PFTs, medical noncompliance, past alcohol and methamphetamine abuse who was brought in from ambulance at the border.

Relevant imaging:

X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
DATE/TIME: 1/12/2024 8:49 PM
IMPRESSION:
Large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly

X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
DATE/TIME: 1/13/2024 6:59 AM
HISTORY: intubation
FINDINGS:
Endotracheal tube terminates 4.6 cm above the carina. Gastric tube terminates in the proximal stomach.
The heart and mediastinum are unchanged.
Unchanged large right pleural effusion with right basilar atelectasis or infiltrate. No pneumothorax.
Imaged skeletal structures are unchanged.
IMPRESSION: Endotracheal and gastric tubes as described below.
Unchanged large right pleural effusion

X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
DATE/TIME: 1/13/2024 12:09 PM
FINDINGS:
New right chest tube. Endotracheal and gastric tubes are in stable position.
The heart and mediastinum are unchanged.
Interval decrease in moderate right pleural effusion. No pneumothorax. Right basilar atelectasis or infiltrate again noted.
Imaged skeletal structures are unchanged.
IMPRESSION: Interval decrease in moderate right pleural effusion status post chest tube insertion. No pneumothorax.

X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW
DATE/TIME: 1/14/2024 4:29 AM
IMPRESSION:
Essentially unchanged examination.

**Pneumonia history: **No

Previous ST evaluation/treatment and/or history of swallow impairments: none per chart

**Prior living situation: **Independent

Assessment
Subjective: Pt's RN Chelsie cleared Pt for BSE and reported that pt may have oral thrush vs oral herpes. Pt refused oral care due to severe oral pain and open sores on tongue, lips, and cheeks.

Current diet: NPO Diet NPO; Ice Chips, Sips With Meds, Sips Of Clear Liquids
Baseline diet texture: Regular solids/all liquids

Respiratory status: Nasal Cannula, 2L
History of recent intubation: Yes: 01/13/2024 to 01/14/2024

Oral Motor Examination:
Dentition: Natural Teeth and Missing Teeth
Vocal quality: clear
Motor Speech: WNL

The patient demonstrated** *impairments*** in the following oral motor areas:
Poor oral condition: open sores on tongue and on cheeks

Overall oral motor: WFL but with open sores

Evaluation
The oropharyngeal swallow was characterized by the following signs and symptoms:

Oral Phase Signs and Symptoms:
Thin liquids via cup, Thin liquids via straw, Dysphagia 1 solids, and Dysphagia 3 solids: Prolonged mastication

Pharyngeal Phase Signs and Symptoms:
Thin liquids via cup, Thin liquids via straw, Dysphagia 1 solids, and Dysphagia 3 solids: None (unable to r/o silent aspiration at bedside)
Pt deferred regular solids due to high levels of pain.

Esophageal Phase Signs and Symptoms:
Belching with thin liquids

Further evaluation recommended to formally and objectively assess swallow function: No

[table#2]:
**IMPRESSIONS AND RECOMMENDATIONS: **

Clinical Impressions: Pt presents with mild oral dysphagia at this time due to pain from open sores. Pharyngeal phase WFL without any overt signs of penetration or aspiration. Pt is appropriate to continue Dysphagia 3 solid/all liquid diet. Regular solids not recommended due to levels of pain when chewing, but pt can likely safely advance to regular solids when oral sores resolve. No further dysphagia tx is indicated at this time.

Risk for aspiration: Low

**FOIS - Functional Oral Intake Scale: **5 - Total oral intake of multiple consistencies requiring special preparation

**Recommendations: **
**Solids: D
ysphagia 3**
Liquids: All liquids
Administer medication: Whole with liquid and One at a time

**Swallow Guidelines: **standard aspiration precautions
Level of Supervision: Independent

**Recommend ST at next level of care: **No
SLP Discharge Recommendations: Independent

**Current treatment plan: **n/a
Recommended referral: None
Prognosis:
Rehabilitation Potential: N/A
Age-Specific Considerations: Adult
Barriers to Learning: None

Education:
Education provided: Diet recommendations, Safety and precautions, Compensatory strategies, Risk of aspiration, Suggested POC, and Need for frequent oral care
Education provided to: Patient and Other: RN, MD
Response: Communicated Understanding and Returned Demonstration

[x] Goals and Plan of care discussed and agreed upon with: pt, RN, MD

**Goals: **
Patient/family stated goals: to eat brunch
[table#3]:
Long Term Goals	**Current Progress Towards Goals**	**Estimated Completion Date**
Patient will safely swallow the least restrictive texture without overt clinical signs/symptoms of laryngeal aspiration or penetration	1/15/2024 Goal Completed	N/a

Patient's needs met and call light left within reach.

Signed:
Rosemarie L Stitzel, SLP

Electronically signed by Rosemarie S at 01/15/2024 1:55 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: updated pt of plan today including diet, CT care, speech eval, turning and repositioning. Discussed about meds and poc. CT remained in continous suction as per order. Patient v/u and agreed. NFT

**Key findings related to barriers to transfer/discharge: clearance **

Patient/Family needs and concerns: none

Electronically signed by Your Scripps Health Team at 01/15/2024 10:20 AM PST
Electronically signed by Your Scripps Health Team at 01/15/2024 10:23 AM PST

Plan of Care - Nurse Esther N - 01/15/2024 12:43 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt is continuing with plan of care. Pt is A/Ox3-4, forgetful, VSS, on 4LNC. NSR. R chest tube in place to suction with yellow drainage. Pt reports severe mouth pain from sores, PRN dilaudid given with some relief. Pt is NPO except sips of clear liquids, meds, ice chips. Pt frequently asks for apple juice and crackers. Per MD ok to give pt crackers, and no IVF. Fall precautions, call light within reach.

0250 Pt complained that she is unable to sleep, notified residents of pt request for sleep medicine.

0300 Per pt mouth sores are severely painful, BLM mouthwash and dilaudid are ineffective. Pt insists that her dentist prescribed her "blistex" and that it was a numbing cream that helped before. Informed pt that blistex is a chapstick. Pt continued to state that blistex is not chapstick and it will numb her mouth. Notified night residents. Gave melatonin for sleep.

0418 Administered lidocaine spray to mouth sores, pt then stated that her neck is stiff and painful and will lead to a migraine, notified residents, ordered lidocaine patch for neck. Pt continues to ask for medicine for sleep. Reminded pt that it was given already.

0439 Applied lidocaine patch to pt's neck. Pt now states that her toes are stiff and painful. Applied hot packs, effective per pt. Pt then asked for lidocaine spray in her mouth. Informed pt that she can have the spray every 6 hours. Pt repeated "can I have the spray?" Reminded pt that she got the spray 10 mins ago.

**Key findings related to barriers to transfer/discharge: **medical clearance, chest tube

Patient/Family needs and concerns: updates PRN

Electronically signed by Nurse Esther N at 01/15/2024 12:48 AM PST
Electronically signed by Nurse Esther N at 01/15/2024 12:48 AM PST
Electronically signed by Nurse Esther N at 01/15/2024 3:23 AM PST
Electronically signed by Nurse Esther N at 01/15/2024 4:22 AM PST
Electronically signed by Nurse Esther N at 01/15/2024 4:36 AM PST
Electronically signed by Nurse Esther N at 01/15/2024 4:44 AM PST
Electronically signed by Nurse Esther N at 01/15/2024 5:09 AM PST
Electronically signed by Nurse Esther N at 01/15/2024 5:15 AM PST

Plan of Care - Your Scripps Health Team - 01/14/2024 7:19 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: received pt at around 1830; aox4; with chest tube to suction 20cm; IV's all SL. Oriented pt to her new environment. Needs attended and met. VSS; afebrile. No pain and no sob reported; on 4liters NC at this time per ICU RN Jocelyn. Discussed poc; skin checked with Analyn B. RN with blancheable redness on sacral; heels, toes, bilateral elbows. Spoke with MD Ho and said no IVF for now d/t CHF. Patient v/u and agreed. 19
22

**Key findings related to barriers to transfer/discharge: clearance **

Patient/Family needs and concerns: none

Electronically signed by Your Scripps Health Team at 01/14/2024 7:22 PM PST

Plan of Care - Your Scripps Health Team - 01/14/2024 1:49 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Intubated hooked to ventilator not in distress. On Propofol and Fentanyl drips.
Fentanyl and Propofol on stand by, patient for SBT.
1115- Patient was doing good with SBT, per Dr. Dammert to extubate the patient. RT extubated the patient and placed on nasal cannula.
1400- Patient c/o pain on her mouth and tongue. Mouth with multiple white spots and small lesions. Examined by Dr. Dammert and ordered diphenhydramine-lidocaine viscous-maalox mouthwash. Patient was relieved.
Patient passed the 3 oz water test and per Dr. Ho okay to give patient crackers only. Given crackers and patient said it hurts her tongue but able to eat crackers, no s/s of aspiration.
Chest tube intact with yellowish output.
For transfer to tele and oral report given to Chelsea, RN

**Key findings related to barriers to transfer/discharge: **Stepdown level of care

Patient/Family needs and concerns: patient wanted to eat

Electronically signed by Your Scripps Health Team at 01/14/2024 5:26 PM PST

OT Patient Not Seen Note - Orson B - 01/14/2024 9:49 AM PST

Formatting of this note might be different from the original.

[image]

Patient Not Seen Notification

Patient Name: Elizabeth Tina Petruccio
Unit/Room: 0ICU/11
**Primary Diagnosis: **The primary encounter diagnosis was Acute respiratory failure with hypercapnia (CMS/HCC). Diagnoses of COPD exacerbation (CMS/HCC), Pleural effusion, Oral thrush, Acute kidney injury (CMS/HCC), Prolonged QT interval, and Loculated pleural effusion were also pertinent to this visit.

**Date: **1/14/2024

**Reason For Deferral: **
[ ] Patient Refused
[ ] Family Deferred
[ ] Therapist Deferred
[ ] RN Deferred
[ ] MD Deferred
[x] Medical Contraindication
[ ] Awaiting Consultation
[ ] Pt Unable to Participate
[ ] Pt Unavailable for Treatment
[ ] Scheduling Conflict
[ ] Patient At or Pending Surgery
[ ] Other:

**Comment: **Patient intubated after OT eval order. OT will need new order to restart therapy.

Orson L. Billenstein, OT
1/14/2024

Electronically signed by Orson B at 01/14/2024 9:49 AM PST

Plan of Care - Your Scripps Health Team - 01/14/2024 7:34 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Pt is responsive to voice and able to squeeze hands/wiggle toes lightly. Pupils are brisk, equal, and reactive. LS clear/dim bilatrally. Pt is on the vent: PRVC: FIO2 40% PEEP 5 RR 20 Tidals 400. ETT is 22 cm @ the teeth. Pt had a small amount of clear thin secretions endotracheally. Pt is in NSR. HR 70's-80's. Pressures 90's-140/60's-70's. Pt's bilateral feet are reddened and the bilateral hands are purplish in color. Pt also has blanchable redness on bilateral elbows - bilateral mepilexes in place. Pt's abdomen is rounded and non-distended. Pt has a purewick in place. Pt was bladder scanned x2 - 483 mL and 214 mL. Pt was straight cathed x1 - 600 mL removed. No bm. Pt has a right-sided midaxillary chest tube w/ a small amount of purulent, milky, yellow output. Pt has a right-sided midline and one right-sided hand PIV. Pt also has a left-sided forearm PIV and a left-sided hand PIV. Pt also has a right-nostril NG tube w/ tube feeds infusing.

Vital AF 1.2 @ 40 mL/hour (50 mL = goal) w/ a FWF @ 30 mL 4H.

Amiodarone (1.8 mg/mL) @ 0.5 mg/min

D5LR @ 30 mL/hr

Fentanyl (50 mcg/mL)@ 50 mcg/hr

Propofol (10 mg/mL) @ 40 mcg/kg/min

Jack Verkuilen - RN

Electronically signed by Your Scripps Health Team at 01/14/2024 7:34 AM PST

Plan of Care - Your Scripps Health Team - 01/13/2024 2:22 PM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events: Intubated hooked to ventilator not in distress. On Fentanyl and Propofol drips.
2Decho done at bedside.
Chest tube inserted by Brandon, NP and Dr. Dammert at bedside. With large amount of yellowish pussy output total of 1400 ml right away. Chest tube connected to 20cm suction.
PCXR done,
Doppler studies both lower extremities done.
Bladder scan 172 ml, per Dr Dammert hold insertion of foley cath for now.
1600- Bladder scan 333
ml
Ultrasound of the liver done at bedside.

Key findings related to barriers to transfer/discharge: ICU level of care

Patient/Family needs and concerns: Daughter Erica updated

Electronically signed by Your Scripps Health Team at 01/13/2024 6:42 PM PST

PT Patient Not Seen - Marie G - 01/13/2024 10:52 AM PST

Formatting of this note might be different from the original.
Physical Therapy

[table#1]:
	01/13/24 1052
Assessment Type
Missed Time Reason	Unable to participate
(Intubated and sedated. PT to follow up on pt status tomorrow.)

Electronically signed by Marie G at 01/13/2024 10:53 AM PST

Plan of Care - Your Scripps Health Team - 01/13/2024 5:37 AM PST

Formatting of this note might be different from the original.
Nursing Summary of Patient Progress

Important trends/observation and significant events:

Pt admitted from Ed/ report and beside handoff by Jess.
**Pt RAAS -3 later restless oriented x3. Abdominal pain which per pt occurred many times in the past. **
Pt so restless gets OOB. Does not tolerate BIPAP. Nausea Zofran given.
Thoracentesis R purulent output .
5.40 ABG declining resp. status. Intubation planned and prepared.

Key findings related to barriers to transfer/discharge: critical ill

Patient/Family needs and concerns: Md tried to reach family

Electronically signed by Your Scripps Health Team at 01/13/2024 5:41 AM PST

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
Pneumococcal 50+ (1 of 2 - PCV)	**07/29/1983**		
Breast Cancer Screening	**07/29/2004**		
Colorectal Cancer Screening	**07/29/2009**		
RSV (1 - Risk 50-74 years 1-dose series)	**07/29/2014**		
Shingrix Vaccine (1 of 2)	**07/29/2014**		
Cervical Cancer Screening	**02/27/2024**	02/27/2019, 01/20/2012	
Flu Shot (#1)	**09/01/2026**	11/20/2012	
Adult Tdap/Td Vaccines (2 - Td or Tdap)	02/09/2034	02/09/2024	

Procedures - documented in this encounter

Procedure Name	Priority	Date/Time	Associated Diagnosis	Comments
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/12/2024 9:50 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/12/2024 12:43 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/11/2024 11:27 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/11/2024 2:04 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/09/2024 11:17 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/09/2024 1:17 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/08/2024 10:52 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/07/2024 10:57 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/07/2024 10:03 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/06/2024 10:34 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/06/2024 10:59 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/05/2024 11:32 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/04/2024 11:08 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/04/2024 10:04 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/04/2024 1:55 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/03/2024 10:18 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/02/2024 11:30 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/01/2024 10:30 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		02/01/2024 2:11 AM PST		
RESPIRATORY THERAPY EVAL AND TREAT PROTOCOL	Routine	01/31/2024 10:38 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/31/2024 7:04 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/31/2024 10:47 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/30/2024 7:17 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/30/2024 7:16 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/30/2024 11:02 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/29/2024 9:31 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/28/2024 8:01 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/28/2024 9:51 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/27/2024 11:32 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/27/2024 9:43 AM PST		

Procedure Name	Priority	Date/Time	Associated Diagnosis	Comments
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/27/2024 3:30 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/26/2024 12:37 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/26/2024 12:25 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/25/2024 3:06 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/24/2024 10:57 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/24/2024 11:23 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/23/2024 10:29 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/23/2024 12:16 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/23/2024 10:38 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/23/2024 12:50 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/22/2024 9:27 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/21/2024 9:52 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/21/2024 9:14 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/20/2024 11:45 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/20/2024 12:54 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/19/2024 10:13 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/18/2024 11:00 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/18/2024 12:38 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/17/2024 12:53 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/17/2024 6:43 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/17/2024 6:42 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/17/2024 4:50 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/16/2024 12:29 PM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/16/2024 12:37 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/16/2024 12:35 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/16/2024 12:30 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/15/2024 9:43 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/14/2024 11:21 PM PST		
EXTUBATION	Routine	01/14/2024 11:13 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/14/2024 7:09 AM PST		
VENTILATOR	Routine	01/13/2024 11:12 PM PST		

Procedure Name	Priority	Date/Time	Associated Diagnosis	Comments
TUBE THORACOSTOMY INCLUDES WATER SEAL	Routine	01/13/2024 11:44 AM PST	Loculated pleural effusion	
Echocardiogram 2D With Definity Protocol As Needed (ECHOCARDIOGRAM 2D (WITH DEFINITY PROTOCOL/COLOR DOPPLER/SPECTRAL DOPPLER/ STRAIN/3D PRN))	Routine	01/13/2024 9:44 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/13/2024 7:47 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/13/2024 7:22 AM PST		
VENTILATOR	Routine	01/13/2024 7:00 AM PST		
VENTILATOR	Routine	01/13/2024 7:00 AM PST		
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)		01/13/2024 4:41 AM PST		

Results - documented in this encounter

Nuc Med hepatobiliary sys without pharm including gallbladder (NUC MED HEPATOBILIARY SYSTEM WITHOUT PHARMACOLOGY) - Final result (02/16/2024 3:20 PM PST)

Anatomical Region	Laterality	Modality
		Nuclear Medicine

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/16/2024 3:16 PM PST	

Impressions

02/16/2024 3:59 PM PST
IMPRESSION: Normal gallbladder visualization. There is no evidence of acute cholecystitis or biliary ductal dilation.

release

Narrative

02/16/2024 3:59 PM PST
EXAMINATION: NUC MED HEPATOBILIARY SYSTEM WITHOUT PHARMACOLOGY

DATE/TIME: 2/16/2024 1:55 PM

HISTORY: RUQ abdominal pain, biliary disease suspected, US nondiagnostic

COMPARISON: Right upper quadrant ultrasound 2/15/2024

TECHNIQUE: Radiopharmaceutical was injected intravenously and sequential static biliary tract imaging was performed.

PHARMACEUTICAL: 5.3 mCi Tc-99m biliary IDA derivative

FINDINGS:
BILIARY DUCTS: Normal radioisotopic biliary excretion.
GALLBLADDER: Normal gallbladder visualization beginning at 25 minutes with no evidence of cystic duct obstruction.
INTESTINE: Normal progression of tracer into the small bowel seen beginning at 35 minutes. No evidence of common biliary ductal obstruction.
OTHER: None.

Procedure Note

Lida Chaipat, MD - 02/16/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: NUC MED HEPATOBILIARY SYSTEM WITHOUT PHARMACOLOGY

DATE/TIME: 2/16/2024 1:55 PM

HISTORY: RUQ abdominal pain, biliary disease suspected, US nondiagnostic

COMPARISON: Right upper quadrant ultrasound 2/15/2024

TECHNIQUE: Radiopharmaceutical was injected intravenously and sequential static biliary tract imaging was performed.

PHARMACEUTICAL: 5.3 mCi Tc-99m biliary IDA derivative

FINDINGS:
BILIARY DUCTS: Normal radioisotopic biliary excretion.
GALLBLADDER: Normal gallbladder visualization beginning at 25 minutes with no evidence of cystic duct obstruction.
INTESTINE: Normal progression of tracer into the small bowel seen beginning at 35 minutes. No evidence of common biliary ductal obstruction.
OTHER: None.

IMPRESSION:
IMPRESSION: Normal gallbladder visualization. There is no evidence of acute cholecystitis or biliary ductal dilation.

release

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	IMG NM PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (02/16/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.0	3.4 - 11.0 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
RBC	3.21	3.98 - 5.25 M/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
HGB	9.7	11.9 - 15.3 g/dL		02/16/2024 6:36 AM PST	MERCY LAB CV	
HCT	32.3	37.3 - 46.7 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/16/2024 6:36 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		02/16/2024 6:36 AM PST	MERCY LAB CV	
MCHC	30.0	31.0 - 36.0 g/dL		02/16/2024 6:36 AM PST	MERCY LAB CV	
RDW	13.4	11.7 - 14.9 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Platelet Count	305	150 - 425 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MPV	8.4	9.0 - 12.8 fL		02/16/2024 6:36 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/16/2024 6:36 AM PST	MERCY LAB CV	
Neutrophils %	38.2	38.0 - 74.0 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
IG%	0.2	0.0 - 1.2 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Lymphocytes %	45.0	16.0 - 48.0 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Monocytes %	8.2	4.9 - 12.5 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Eosinophils %	7.4	0.0 - 9.5 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Neutrophils	1.91	1.50 - 7.40 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
IG#	0.01	0.00 - 0.10 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.25	0.90 - 3.10 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Monocytes	0.41	0.26 - 0.87 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.37	0.00 - 0.51 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	
Preliminary Abs Neut	1.91	1.50 - 7.40 K/mcL		02/16/2024 6:36 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/16/2024 5:45 AM PST	02/16/2024 6:02 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/16/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/16/2024 6:24 AM PST	MERCY LAB CV	
Glucose	113	70 - 125 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Sodium	137	137 - 145 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Urea Nitrogen	23	7 - 17 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		02/16/2024 6:24 AM PST	MERCY LAB CV	
Anion Gap	6	6 - 14 mmol/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Osmolality Calculated	288	280 - 305 mOsm/kg H20		02/16/2024 6:24 AM PST	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Protein, total	8.4	6.3 - 8.2 g/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	
Alkaline Phosphatase	80	38 - 126 Units/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
AST	38	14 - 36 Units/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
ALT	26	<=34 Units/L		02/16/2024 6:24 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/16/2024 6:24 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/16/2024 5:45 AM PST	02/16/2024 6:02 AM PST

Narrative

MERCY LAB CV - 02/16/2024 6:24 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

Ultrasound abdominal limited gallbladder (ULTRASOUND ABDOMEN LIMITED GALLBLADDER) - Final result (02/15/2024 6:30 PM PST)

Anatomical Region	Laterality	Modality
Body		Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/15/2024 6:58 PM PST	

Impressions

02/15/2024 7:01 PM PST
IMPRESSION:
1. Tumefactive sludge favored over nonshadowing new gallstones compared to negative gallbladder study of 1/13/2024.
2. Slight gallbladder wall thickening without pericholecystic edema or sonographic Murphy sign not so pronounced to confirm acute cholecystitis. Nuclear medicine biliary scan can clarify if additional imaging indicated clinically.
3. Borderline hepatomegaly with slightly echogenic dense sonographic pattern suspicious for mild hepatic steatosis. No surface contour abnormality, or ascites to suggest cirrhosis. No visible lesion to suggest infiltrative malignancy.

release

Narrative

02/15/2024 7:01 PM PST

Narrative

EXAMINATION: ULTRASOUND ABDOMEN LIMITED GALLBLADDER

DATE/TIME: 2/15/2024 6:30 PM

HISTORY: RUQ pain

ADDITIONAL CLINICAL INFORMATION: CHF.

COMPARISON: 1/13/2024.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: The echogenicity of the liver is increased. Liver is mildly enlarged at 17.4 cm cephalocaudad reaching well below the lower pole the right kidney. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
GALLBLADDER: The gallbladder now contains echogenic debris, nonshadowing. This most likely is related to tumefactive sludge rather than nonshadowing gallstones. Filled biliary interval dilatation of the common bile duct to 8.2 mm without visible calcified common duct stone or mass. No intra paddock duct dilatation.
BILIARY: No ductal dilatation.
PANCREAS: Visualized pancreatic head and body negative for focal mass or ductal dilatation.
RIGHT KIDNEY: No visible solid mass, calculus or hydronephrosis.
AORTA/VASC: No visible aneurysm.
IVC: The visible IVC is patent.
OTHER: No free fluid.

The following measurements were obtained:
AORTA PROXIMAL: 2.1 cm
AORTA DISTAL: 1.5 cm
LIVER: 17.4 cm
GALLBLADDER WALL: 3.2 mm
CBD: 8.2 mm
RT KIDNEY: 10.4 cm x 4.4 cm x 5.3 cm

Procedure Note

Kris Vanlom, MD - 02/15/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: ULTRASOUND ABDOMEN LIMITED GALLBLADDER

DATE/TIME: 2/15/2024 6:30 PM

HISTORY: RUQ pain

ADDITIONAL CLINICAL INFORMATION: CHF.

COMPARISON: 1/13/2024.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: The echogenicity of the liver is increased. Liver is mildly enlarged at 17.4 cm cephalocaudad reaching well below the lower pole the right kidney. No visible mass.
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GALLBLADDER: The gallbladder now contains echogenic debris, nonshadowing. This most likely is related to tumefactive sludge rather than nonshadowing gallstones. Filled biliary interval dilatation of the common bile duct to 8.2 mm without visible calcified common duct stone or mass. No intra paddock duct dilatation.
BILIARY: No ductal dilatation.
PANCREAS: Visualized pancreatic head and body negative for focal mass or ductal dilatation.
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AORTA/VASC: No visible aneurysm.
IVC: The visible IVC is patent.
OTHER: No free fluid.

The following measurements were obtained:
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LIVER: 17.4 cm
GALLBLADDER WALL: 3.2 mm
CBD: 8.2 mm
RT KIDNEY: 10.4 cm x 4.4 cm x 5.3 cm

IMPRESSION:
IMPRESSION:
1. Tumefactive sludge favored over nonshadowing new gallstones compared to negative gallbladder study of 1/13/2024.
2. Slight gallbladder wall thickening without pericholecystic edema or sonographic Murphy sign not so pronounced to confirm acute cholecystitis. Nuclear medicine biliary scan can clarify if additional imaging indicated clinically.
3. Borderline hepatomegaly with slightly echogenic dense sonographic pattern suspicious for mild hepatic steatosis. No surface contour abnormality, or ascites to suggest cirrhosis. No visible lesion to suggest infiltrative malignancy.

release

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	IMG US PROCEDURES	Final Result

C-REACTIVE PROTEIN - Final result (02/14/2024 11:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	<5.0	<10.0 mg/L		02/14/2024 11:45 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/14/2024 11:11 AM PST	02/14/2024 11:24 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (02/14/2024 11:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	111	<=30 mm/hr		02/14/2024 1:44 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/14/2024 11:11 AM PST	02/14/2024 11:27 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/14/2024 5:47 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	104	70 - 125 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Sodium	137	137 - 145 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
CO2, Total	33	22 - 30 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Creatinine	0.96	0.50 - 1.00 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	68	>60 mL/min/1.73m*2		02/14/2024 6:24 AM PST	MERCY LAB CV	
Anion Gap	3	6 - 14 mmol/L		02/14/2024 6:24 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality Calculated	287	280 - 305 mOsm/kg H20		02/14/2024 6:24 AM PST	MERCY LAB CV	
Calcium	8.9	8.4 - 10.2 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Phosphorus	5.6	2.5 - 4.5 mg/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	
Albumin	3.4	3.5 - 5.0 g/dL		02/14/2024 6:24 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/14/2024 5:47 AM PST	02/14/2024 6:00 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/13/2024 7:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
RBC	3.20	3.98 - 5.25 M/mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
HGB	9.8	11.9 - 15.3 g/dL		02/13/2024 8:01 AM PST	MERCY LAB CV	
HCT	32.2	37.3 - 46.7 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/13/2024 8:01 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/13/2024 8:01 AM PST	MERCY LAB CV	
MCHC	30.4	31.0 - 36.0 g/dL		02/13/2024 8:01 AM PST	MERCY LAB CV	
RDW	13.8	11.7 - 14.9 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Platelet Count	333	150 - 425 K/mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
MPV	8.3	9.0 - 12.8 fL		02/13/2024 8:01 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
nRBC	0.4	0.0 /100 WBCs		02/13/2024 8:01 AM PST	MERCY LAB CV	
Neutrophils %	44.1	38.0 - 74.0 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
IG%	0.4	0.0 - 1.2 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Lymphocytes %	40.9	16.0 - 48.0 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Monocytes %	8.9	4.9 - 12.5 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Eosinophils %	4.9	0.0 - 9.5 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.35	1.50 - 7.40 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.17	0.90 - 3.10 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Monocytes	0.47	0.26 - 0.87 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.26	0.00 - 0.51 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.35	1.50 - 7.40 K/ mcL		02/13/2024 8:01 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/13/2024 7:36 AM PST	02/13/2024 7:40 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/13/2024 7:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	95	70 - 125 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Sodium	139	137 - 145 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Urea Nitrogen	22	7 - 17 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		02/13/2024 8:03 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Osmolality Calculated	291	280 - 305 mOsm/kg H20		02/13/2024 8:03 AM PST	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Phosphorus	5.1	2.5 - 4.5 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/13/2024 7:36 AM PST	02/13/2024 7:40 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/13/2024 7:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/13/2024 8:03 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	95	70 - 125 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Sodium	139	137 - 145 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Urea Nitrogen	22	7 - 17 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		02/13/2024 8:03 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Osmolality Calculated	291	280 - 305 mOsm/kg H20		02/13/2024 8:03 AM PST	MERCY LAB CV	
Calcium	9.1	8.4 - 10.2 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Protein, total	8.4	6.3 - 8.2 g/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Albumin	3.8	3.5 - 5.0 g/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	
Alkaline Phosphatase	79	38 - 126 Units/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
AST	35	14 - 36 Units/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
ALT	25	<=34 Units/L		02/13/2024 8:03 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/13/2024 8:03 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/13/2024 7:36 AM PST	02/13/2024 7:40 AM PST

Narrative

MERCY LAB CV - 02/13/2024 8:03 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/12/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	97	70 - 125 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Urea Nitrogen	21	7 - 17 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Creatinine	0.81	0.50 - 1.00 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	84	>60 mL/min/1.73m*2		02/12/2024 6:37 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/12/2024 6:37 AM PST	MERCY LAB CV	
Osmolality Calculated	289	280 - 305 mOsm/kg H20		02/12/2024 6:37 AM PST	MERCY LAB CV	
Calcium	9.0	8.4 - 10.2 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Phosphorus	5.3	2.5 - 4.5 mg/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	
Albumin	3.5	3.5 - 5.0 g/dL		02/12/2024 6:37 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/12/2024 5:28 AM PST	02/12/2024 6:12 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (02/11/2024 8:09 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	113	70 - 125 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Urea Nitrogen	23	7 - 17 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Creatinine	0.84	0.50 - 1.00 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	80	>60 mL/min/1.73m*2		02/11/2024 8:46 PM PST	MERCY LAB CV	
Anion Gap	4	6 - 14 mmol/L		02/11/2024 8:46 PM PST	MERCY LAB CV	
Osmolality Calculated	290	280 - 305 mOsm/kg H20		02/11/2024 8:46 PM PST	MERCY LAB CV	
Calcium	8.8	8.4 - 10.2 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Phosphorus	4.3	2.5 - 4.5 mg/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	
Albumin	3.6	3.5 - 5.0 g/dL		02/11/2024 8:46 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/11/2024 8:09 PM PST	02/11/2024 8:20 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray hip with and without pelvis right 2-3 views (X-RAY HIP RIGHT 2-3 VIEWS) - Final result (02/11/2024 4:26 PM PST)

Anatomical Region	Laterality	Modality
Hip	Right	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/11/2024 4:42 PM PST	

Impressions

02/12/2024 11:04 AM PST
IMPRESSION:
1. Mild degenerative changes at right hip joint. No acute fracture detected.

release

Narrative

02/12/2024 11:04 AM PST
EXAMINATION: X-RAY HIP RIGHT 2-3 VIEWS

DATE/TIME: 2/11/2024 4:26 PM

HISTORY: Right hip pain

COMPARISON: None.

FINDINGS:

Two upright views of the right hip obtained.

There is normal alignment of the right hip joint without dislocation. There is mild superior joint space narrowing at the right hip joint. Minimal degenerative spurring seen. No plain film evident acute fracture at the right hip detected.

There is normal contour and density of the right femoral head.

Mild degenerative changes of the pubic symphysis seen.

Procedure Note

Jerome Pierce, MD - 02/12/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY HIP RIGHT 2-3 VIEWS

DATE/TIME: 2/11/2024 4:26 PM

HISTORY: Right hip pain

COMPARISON: None.

FINDINGS:

Two upright views of the right hip obtained.

There is normal alignment of the right hip joint without dislocation. There is mild superior joint space narrowing at the right hip joint. Minimal degenerative spurring seen. No plain film evident acute fracture at the right hip detected.

There is normal contour and density of the right femoral head.

Mild degenerative changes of the pubic symphysis seen.

IMPRESSION:
IMPRESSION:
1. Mild degenerative changes at right hip joint. No acute fracture detected.

release

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (02/11/2024 4:08 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/11/2024 4:11 PM PST	

Impressions

02/11/2024 4:11 PM PST
IMPRESSION: No deep venous thrombosis detected in either leg.

release

Narrative

02/11/2024 4:11 PM PST

Narrative

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 2/11/2024 4:08 PM

HISTORY: Dvt

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable. The technologist could not see the left peroneal vein possibly due to presence of edema.
OTHER: None.

Procedure Note

Jerome Pierce, MD - 02/11/2024

Formatting of this note might be different from the original.

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 2/11/2024 4:08 PM

HISTORY: Dvt

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable. The technologist could not see the left peroneal vein possibly due to presence of edema.
OTHER: None.

IMPRESSION:
IMPRESSION: No deep venous thrombosis detected in either leg.

release

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	IMG US PROCEDURES	Final Result

12-LEAD ECG - Final result (02/11/2024 10:50 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	98	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	98	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	184	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	90	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	386	ms			SCRIPPS RADIOLOGY	
QTC	492	ms			SCRIPPS RADIOLOGY	
P AXIS	25	degrees			SCRIPPS RADIOLOGY	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
R AXIS	55	degrees			SCRIPPS RADIOLOGY	
T AXIS	133	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/11/2024 10:50 AM PST	02/23/2024 3:34 PM PST

Impressions

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SCRIPPS RADIOLOGY - 02/23/2024 3:34 PM PST
Normal sinus rhythm
ST & T wave abnormality, consider lateral ischemia
Abnormal ECG
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Narrative

This result has an attachment that is not available.

Procedure Note

Raghava Gollapudi, MD - 02/23/2024

Formatting of this note might be different from the original.

IMPRESSION:
Normal sinus rhythm
ST & T wave abnormality, consider lateral ischemia
Abnormal ECG

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

CBC WITH DIFFERENTIAL - Final result (02/10/2024 5:52 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	6.0	3.4 - 11.0 K/mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
RBC	3.25	3.98 - 5.25 M/mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
HGB	10.1	11.9 - 15.3 g/dL		02/10/2024 6:39 AM PST	MERCY LAB CV	
HCT	32.4	37.3 - 46.7 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/10/2024 6:39 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/10/2024 6:39 AM PST	MERCY LAB CV	
MCHC	31.2	31.0 - 36.0 g/dL		02/10/2024 6:39 AM PST	MERCY LAB CV	
RDW	13.8	11.7 - 14.9 %		02/10/2024 6:39 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	377	150 - 425 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
MPV	8.4	9.0 - 12.8 fL		02/10/2024 6:39 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/10/2024 6:39 AM PST	MERCY LAB CV	
Neutrophils %	45.9	38.0 - 74.0 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
IG%	0.3	0.0 - 1.2 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Lymphocytes %	41.3	16.0 - 48.0 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Monocytes %	8.2	4.9 - 12.5 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Eosinophils %	3.5	0.0 - 9.5 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.74	1.50 - 7.40 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.47	0.90 - 3.10 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Monocytes	0.49	0.26 - 0.87 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.21	0.00 - 0.51 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.74	1.50 - 7.40 K/ mcL		02/10/2024 6:39 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/10/2024 5:52 AM PST	02/10/2024 6:32 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/10/2024 5:52 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/10/2024 7:00 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Urea Nitrogen	21	7 - 17 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/10/2024 7:00 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Osmolality Calculated	277	280 - 305 mOsm/kg H20		02/10/2024 7:00 AM PST	MERCY LAB CV	
Calcium	9.0	8.4 - 10.2 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Protein, total	8.5	6.3 - 8.2 g/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Albumin	3.6	3.5 - 5.0 g/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	
Alkaline Phosphatase	80	38 - 126 Units/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
AST	33	14 - 36 Units/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
ALT	21	<=34 Units/L		02/10/2024 7:00 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		02/10/2024 7:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/10/2024 5:52 AM PST	02/10/2024 6:31 AM PST

Narrative

MERCY LAB CV - 02/10/2024 7:00 AM PST

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for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (02/09/2024 10:58 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/09/2024 11:35 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	
Sodium	136	137 - 145 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
CO2, Total	31	22 - 30 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Urea Nitrogen	22	7 - 17 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	
Creatinine	1.00	0.50 - 1.00 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	65	>60 mL/min/1.73m*2		02/09/2024 11:35 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		02/09/2024 11:35 AM PST	MERCY LAB CV	
Osmolality Calculated	285	280 - 305 mOsm/kg H20		02/09/2024 11:35 AM PST	MERCY LAB CV	
Calcium	8.9	8.4 - 10.2 mg/dL		02/09/2024 11:35 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/09/2024 10:58 AM PST	02/09/2024 11:18 AM PST

Narrative

MERCY LAB CV - 02/09/2024 11:35 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray ankle left 3 or more views (X-RAY ANKLE LEFT 3 OR MORE VIEWS) - Final result (02/07/2024 5:27 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities, Ankle	Left	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/07/2024 5:56 PM PST	

Impressions

02/07/2024 5:57 PM PST
IMPRESSION:
1. Severe soft tissue swelling around the ankle midfoot and forefoot correlate clinically for cellulitis. No radiographic evidence of osteomyelitis. Midfoot degenerative changes

2. No soft tissue emphysema

release

Narrative

02/07/2024 5:57 PM PST
EXAMINATION: X-RAY ANKLE LEFT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle pain, no prior imaging

COMPARISON: None.

FINDINGS: See impression.

Procedure Note

George Wesbey, MD - 02/07/2024

Procedure Note

Formatting of this note might be different from the original.
EXAMINATION: X-RAY ANKLE LEFT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle pain, no prior imaging

COMPARISON: None.

FINDINGS: See impression.

IMPRESSION:
IMPRESSION:
1. Severe soft tissue swelling around the ankle midfoot and forefoot correlate clinically for cellulitis. No radiographic evidence of osteomyelitis. Midfoot degenerative changes

2. No soft tissue emphysema

release

Authorizing Provider	Result Type	Result Status
Howie Que DPM	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

X-ray ankle right 3 or more views (X-RAY ANKLE RIGHT 3 OR MORE VIEWS) - Final result (02/07/2024 5:27 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities, Ankle	Right	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/07/2024 5:57 PM PST	

Impressions

02/07/2024 5:58 PM PST
IMPRESSION:
1. Marked soft tissue swelling at the ankle and hindfoot and midfoot and forefoot

2. Small plantar calcaneal spur

3. No fractures or dislocations identified

release

Narrative

02/07/2024 5:58 PM PST
EXAMINATION: X-RAY ANKLE RIGHT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle trauma, fracture, xray done (Age >= 5y)

COMPARISON: None.

FINDINGS: See impression.

Procedure Note

George Wesbey, MD - 02/07/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY ANKLE RIGHT 3 OR MORE VIEWS

DATE/TIME: 2/7/2024 5:27 PM

HISTORY: Ankle trauma, fracture, xray done (Age >= 5y)

COMPARISON: None.

FINDINGS: See impression.

IMPRESSION:
IMPRESSION:
1. Marked soft tissue swelling at the ankle and hindfoot and midfoot and forefoot

2. Small plantar calcaneal spur

3. No fractures or dislocations identified

release

Authorizing Provider	Result Type	Result Status
Howie Que DPM	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (02/07/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
RBC	2.96	3.98 - 5.25 M/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
HGB	9.2	11.9 - 15.3 g/dL		02/07/2024 6:51 AM PST	MERCY LAB CV	
HCT	29.5	37.3 - 46.7 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/07/2024 6:51 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/07/2024 6:51 AM PST	MERCY LAB CV	
MCHC	31.2	31.0 - 36.0 g/dL		02/07/2024 6:51 AM PST	MERCY LAB CV	
RDW	14.1	11.7 - 14.9 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Platelet Count	446	150 - 425 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
MPV	8.3	9.0 - 12.8 fL		02/07/2024 6:51 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/07/2024 6:51 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Neutrophils %	44.2	38.0 - 74.0 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Lymphocytes %	41.7	16.0 - 48.0 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Monocytes %	9.2	4.9 - 12.5 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Eosinophils %	3.2	0.0 - 9.5 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Basophils %	0.9	0.0 - 1.6 %		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.34	1.50 - 7.40 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.21	0.90 - 3.10 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Monocytes	0.49	0.26 - 0.87 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.17	0.00 - 0.51 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.34	1.50 - 7.40 K/mcL		02/07/2024 6:51 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/07/2024 5:35 AM PST	02/07/2024 6:34 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/07/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/07/2024 7:34 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	94	70 - 125 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/07/2024 7:34 AM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Osmolality Calculated	274	280 - 305 mOsm/kg H20		02/07/2024 7:34 AM PST	MERCY LAB CV	
Calcium	8.8	8.4 - 10.2 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Protein, total	8.0	6.3 - 8.2 g/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Albumin	3.3	3.5 - 5.0 g/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	
Alkaline Phosphatase	90	38 - 126 Units/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
AST	32	14 - 36 Units/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
ALT	21	<=34 Units/L		02/07/2024 7:34 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		02/07/2024 7:34 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/07/2024 5:35 AM PST	02/07/2024 6:34 AM PST

Narrative

MERCY LAB CV - 02/07/2024 7:34 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/06/2024 5:29 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.0	3.4 - 11.0 K/mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
RBC	3.06	3.98 - 5.25 M/mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
HGB	9.1	11.9 - 15.3 g/dL		02/06/2024 6:32 AM PST	MERCY LAB CV	
HCT	30.5	37.3 - 46.7 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/06/2024 6:32 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		02/06/2024 6:32 AM PST	MERCY LAB CV	
MCHC	29.8	31.0 - 36.0 g/dL		02/06/2024 6:32 AM PST	MERCY LAB CV	
RDW	14.2	11.7 - 14.9 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Platelet Count	472	150 - 425 K/mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
MPV	8.2	9.0 - 12.8 fL		02/06/2024 6:32 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/06/2024 6:32 AM PST	MERCY LAB CV	
Neutrophils %	42.6	38.0 - 74.0 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
IG%	0.6	0.0 - 1.2 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Lymphocytes %	42.5	16.0 - 48.0 %		02/06/2024 6:32 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	10.5	4.9 - 12.5 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Eosinophils %	2.8	0.0 - 9.5 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.11	1.50 - 7.40 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.11	0.90 - 3.10 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Monocytes	0.52	0.26 - 0.87 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.14	0.00 - 0.51 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.11	1.50 - 7.40 K/ mcL		02/06/2024 6:32 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/06/2024 5:29 AM PST	02/06/2024 6:04 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/06/2024 5:29 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/06/2024 6:36 AM PST	MERCY LAB CV	
Glucose	103	70 - 125 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Potassium	4.1	3.5 - 5.1 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	103	98 - 107 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
CO2, Total	25	22 - 30 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Creatinine	0.71	0.50 - 1.00 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	98	>60 mL/min/1.73m*2		02/06/2024 6:36 AM PST	MERCY LAB CV	
Anion Gap	4	6 - 14 mmol/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Osmolality Calculated	275	280 - 305 mOsm/kg H20		02/06/2024 6:36 AM PST	MERCY LAB CV	
Calcium	8.8	8.4 - 10.2 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Protein, total	7.9	6.3 - 8.2 g/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Albumin	3.3	3.5 - 5.0 g/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	
Alkaline Phosphatase	84	38 - 126 Units/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
AST	36	14 - 36 Units/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
ALT	26	<=34 Units/L		02/06/2024 6:36 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		02/06/2024 6:36 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/06/2024 5:29 AM PST	02/06/2024 6:03 AM PST

Narrative

MERCY LAB CV - 02/06/2024 6:36 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/05/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
RBC	2.88	3.98 - 5.25 M/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
HGB	8.8	11.9 - 15.3 g/dL		02/05/2024 7:25 AM PST	MERCY LAB CV	
HCT	29.0	37.3 - 46.7 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/05/2024 7:25 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/05/2024 7:25 AM PST	MERCY LAB CV	
MCHC	30.3	31.0 - 36.0 g/dL		02/05/2024 7:25 AM PST	MERCY LAB CV	
RDW	13.9	11.7 - 14.9 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Platelet Count	515	150 - 425 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
MPV	8.1	9.0 - 12.8 fL		02/05/2024 7:25 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/05/2024 7:25 AM PST	MERCY LAB CV	
Neutrophils %	49.7	38.0 - 74.0 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Lymphocytes %	35.5	16.0 - 48.0 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Monocytes %	10.2	4.9 - 12.5 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Eosinophils %	3.0	0.0 - 9.5 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.64	1.50 - 7.40 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	1.88	0.90 - 3.10 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Monocytes	0.54	0.26 - 0.87 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.16	0.00 - 0.51 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.64	1.50 - 7.40 K/mcL		02/05/2024 7:25 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/05/2024 5:53 AM PST	02/05/2024 6:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/05/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/05/2024 7:10 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Chloride	98	98 - 107 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Urea Nitrogen	15	7 - 17 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Creatinine	0.75	0.50 - 1.00 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	92	>60 mL/min/1.73m*2		02/05/2024 7:10 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Anion Gap	4	6 - 14 mmol/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Osmolality Calculated	273	280 - 305 mOsm/kg H20		02/05/2024 7:10 AM PST	MERCY LAB CV	
Calcium	8.7	8.4 - 10.2 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Protein, total	8.0	6.3 - 8.2 g/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Albumin	3.2	3.5 - 5.0 g/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	
Alkaline Phosphatase	107	38 - 126 Units/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
AST	39	14 - 36 Units/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
ALT	22	<=34 Units/L		02/05/2024 7:10 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/05/2024 7:10 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/05/2024 5:53 AM PST	02/05/2024 6:43 AM PST

Narrative

MERCY LAB CV - 02/05/2024 7:10 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/04/2024 4:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.1	3.4 - 11.0 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
RBC	2.67	3.98 - 5.25 M/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HGB	8.0	11.9 - 15.3 g/dL		02/04/2024 5:35 AM PST	MERCY LAB CV	
HCT	27.2	37.3 - 46.7 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		02/04/2024 5:35 AM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		02/04/2024 5:35 AM PST	MERCY LAB CV	
MCHC	29.4	31.0 - 36.0 g/dL		02/04/2024 5:35 AM PST	MERCY LAB CV	
RDW	13.9	11.7 - 14.9 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Platelet Count	461	150 - 425 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		02/04/2024 5:35 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/04/2024 5:35 AM PST	MERCY LAB CV	
Neutrophils %	44.3	38.0 - 74.0 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
IG%	1.2	0.0 - 1.2 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Lymphocytes %	38.5	16.0 - 48.0 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Monocytes %	10.8	4.9 - 12.5 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Eosinophils %	4.2	0.0 - 9.5 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Neutrophils	1.81	1.50 - 7.40 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
IG#	0.05	0.00 - 0.10 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.57	0.90 - 3.10 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Monocytes	0.44	0.26 - 0.87 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.17	0.00 - 0.51 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	
Preliminary Abs Neut	1.81	1.50 - 7.40 K/mcL		02/04/2024 5:35 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/04/2024 4:11 AM PST	02/04/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/04/2024 4:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/04/2024 6:47 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Potassium	3.9	3.5 - 5.1 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/04/2024 6:47 AM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Osmolality Calculated	273	280 - 305 mOsm/kg H20		02/04/2024 6:47 AM PST	MERCY LAB CV	
Calcium	8.7	8.4 - 10.2 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protein, total	7.3	6.3 - 8.2 g/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Albumin	2.9	3.5 - 5.0 g/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	
Alkaline Phosphatase	81	38 - 126 Units/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
AST	38	14 - 36 Units/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
ALT	22	<=34 Units/L		02/04/2024 6:47 AM PST	MERCY LAB CV	
Bilirubin, Total	<0.1	0.2 - 1.3 mg/dL		02/04/2024 6:47 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/04/2024 4:11 AM PST	02/04/2024 5:24 AM PST

Narrative

MERCY LAB CV - 02/04/2024 6:47 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/03/2024 5:42 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.5	3.4 - 11.0 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
RBC	2.62	3.98 - 5.25 M/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
HGB	8.0	11.9 - 15.3 g/dL		02/03/2024 6:07 AM PST	MERCY LAB CV	
HCT	26.4	37.3 - 46.7 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/03/2024 6:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCH	31	26 - 33 pg		02/03/2024 6:07 AM PST	MERCY LAB CV	
MCHC	30.3	31.0 - 36.0 g/dL		02/03/2024 6:07 AM PST	MERCY LAB CV	
RDW	14.1	11.7 - 14.9 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Platelet Count	496	150 - 425 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
MPV	7.7	9.0 - 12.8 fL		02/03/2024 6:07 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/03/2024 6:07 AM PST	MERCY LAB CV	
Neutrophils %	52.8	38.0 - 74.0 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
IG%	1.3	0.0 - 1.2 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Lymphocytes %	32.7	16.0 - 48.0 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Monocytes %	8.5	4.9 - 12.5 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Eosinophils %	4.0	0.0 - 9.5 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.36	1.50 - 7.40 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
IG#	0.06	0.00 - 0.10 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.46	0.90 - 3.10 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.38	0.26 - 0.87 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.18	0.00 - 0.51 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.36	1.50 - 7.40 K/mcL		02/03/2024 6:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/03/2024 5:42 AM PST	02/03/2024 5:48 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/03/2024 5:42 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/03/2024 6:56 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
CO2, Total	29	22 - 30 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Urea Nitrogen	19	7 - 17 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Creatinine	0.82	0.50 - 1.00 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	82	>60 mL/min/1.73m*2		02/03/2024 6:56 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Osmolality Calculated	274	280 - 305 mOsm/kg H20		02/03/2024 6:56 AM PST	MERCY LAB CV	
Calcium	8.6	8.4 - 10.2 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Protein, total	7.5	6.3 - 8.2 g/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Albumin	3.0	3.5 - 5.0 g/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	
Alkaline Phosphatase	76	38 - 126 Units/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
AST	42	14 - 36 Units/L		02/03/2024 6:56 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ALT	24	<=34 Units/L		02/03/2024 6:56 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		02/03/2024 6:56 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/03/2024 5:42 AM PST	02/03/2024 5:48 AM PST

Narrative

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MERCY LAB CV - 02/03/2024 6:56 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/02/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.3	3.4 - 11.0 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
RBC	2.59	3.98 - 5.25 M/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
HGB	7.9	11.9 - 15.3 g/dL		02/02/2024 5:47 AM PST	MERCY LAB CV	
HCT	26.2	37.3 - 46.7 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		02/02/2024 5:47 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/02/2024 5:47 AM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		02/02/2024 5:47 AM PST	MERCY LAB CV	
RDW	13.9	11.7 - 14.9 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Platelet Count	513	150 - 425 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MPV	7.8	9.0 - 12.8 fL		02/02/2024 5:47 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/02/2024 5:47 AM PST	MERCY LAB CV	
Neutrophils %	44.9	38.0 - 74.0 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
IG%	1.1	0.0 - 1.2 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Lymphocytes %	39.9	16.0 - 48.0 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Monocytes %	8.6	4.9 - 12.5 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Eosinophils %	4.4	0.0 - 9.5 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Basophils %	1.1	0.0 - 1.6 %		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.36	1.50 - 7.40 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
IG#	0.06	0.00 - 0.10 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.10	0.90 - 3.10 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Monocytes	0.45	0.26 - 0.87 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.23	0.00 - 0.51 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Absolute Basophils	0.06	0.00 - 0.09 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.36	1.50 - 7.40 K/mcL		02/02/2024 5:47 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/02/2024 5:21 AM PST	02/02/2024 5:34 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/02/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/02/2024 5:50 AM PST	MERCY LAB CV	
Glucose	88	70 - 125 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Sodium	130	137 - 145 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Urea Nitrogen	19	7 - 17 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Creatinine	0.87	0.50 - 1.00 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	77	>60 mL/min/1.73m*2		02/02/2024 5:50 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Osmolality Calculated	272	280 - 305 mOsm/kg H20		02/02/2024 5:50 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Protein, total	7.2	6.3 - 8.2 g/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	
Alkaline Phosphatase	71	38 - 126 Units/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
AST	28	14 - 36 Units/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		02/02/2024 5:50 AM PST	MERCY LAB CV	
Bilirubin, Total	<0.1	0.2 - 1.3 mg/dL		02/02/2024 5:50 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/02/2024 5:21 AM PST	02/02/2024 5:33 AM PST

Narrative

MERCY LAB CV - 02/02/2024 5:50 AM PST

Narrative

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for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (02/01/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.2	3.4 - 11.0 K/mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
RBC	2.45	3.98 - 5.25 M/mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
HGB	7.5	11.9 - 15.3 g/dL		02/01/2024 6:49 AM PST	MERCY LAB CV	
HCT	24.6	37.3 - 46.7 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		02/01/2024 6:49 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		02/01/2024 6:49 AM PST	MERCY LAB CV	
MCHC	30.5	31.0 - 36.0 g/dL		02/01/2024 6:49 AM PST	MERCY LAB CV	
RDW	13.5	11.7 - 14.9 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Platelet Count	573	150 - 425 K/mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		02/01/2024 6:49 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		02/01/2024 6:49 AM PST	MERCY LAB CV	
Neutrophils %	44.4	38.0 - 74.0 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
IG%	1.0	0.0 - 1.2 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Lymphocytes %	39.6	16.0 - 48.0 %		02/01/2024 6:49 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	9.8	4.9 - 12.5 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Eosinophils %	4.2	0.0 - 9.5 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.33	1.50 - 7.40 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
IG#	0.05	0.00 - 0.10 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.07	0.90 - 3.10 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Monocytes	0.51	0.26 - 0.87 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.22	0.00 - 0.51 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.33	1.50 - 7.40 K/ mcL		02/01/2024 6:49 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/01/2024 5:45 AM PST	02/01/2024 6:32 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (02/01/2024 5:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				02/01/2024 7:09 AM PST	MERCY LAB CV	
Glucose	90	70 - 125 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Sodium	135	137 - 145 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	102	98 - 107 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Urea Nitrogen	17	7 - 17 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		02/01/2024 7:09 AM PST	MERCY LAB CV	
Anion Gap	-3	6 - 14 mmol/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Osmolality Calculated	281	280 - 305 mOsm/kg H20		02/01/2024 7:09 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Protein, total	6.8	6.3 - 8.2 g/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	
Alkaline Phosphatase	71	38 - 126 Units/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
AST	28	14 - 36 Units/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		02/01/2024 7:09 AM PST	MERCY LAB CV	
Bilirubin, Total	<0.1	0.2 - 1.3 mg/dL		02/01/2024 7:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	02/01/2024 5:45 AM PST	02/01/2024 6:32 AM PST

Narrative

MERCY LAB CV - 02/01/2024 7:09 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/31/2024 6:10 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/31/2024 7:09 AM PST	MERCY LAB CV	
Glucose	92	70 - 125 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Sodium	135	137 - 145 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/31/2024 7:09 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Osmolality Calculated	282	280 - 305 mOsm/kg H20		01/31/2024 7:09 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Protein, total	7.3	6.3 - 8.2 g/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Albumin	3.0	3.5 - 5.0 g/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	
Alkaline Phosphatase	88	38 - 126 Units/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
AST	32	14 - 36 Units/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
ALT	17	<=34 Units/L		01/31/2024 7:09 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/31/2024 7:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:10 AM PST	01/31/2024 6:38 AM PST

Narrative

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MERCY LAB CV - 01/31/2024 7:09 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.6	3.4 - 11.0 K/mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
RBC	2.57	3.98 - 5.25 M/mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
HGB	8.0	11.9 - 15.3 g/dL		01/31/2024 7:35 AM PST	MERCY LAB CV	
HCT	26.1	37.3 - 46.7 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/31/2024 7:35 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/31/2024 7:35 AM PST	MERCY LAB CV	
MCHC	30.7	31.0 - 36.0 g/dL		01/31/2024 7:35 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Platelet Count	598	150 - 425 K/mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		01/31/2024 7:35 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/31/2024 7:35 AM PST	MERCY LAB CV	
Neutrophils %	45.8	38.0 - 74.0 %		01/31/2024 7:35 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
IG%	0.5	0.0 - 1.2 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Lymphocytes %	39.7	16.0 - 48.0 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Monocytes %	7.9	4.9 - 12.5 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Eosinophils %	5.0	0.0 - 9.5 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Basophils %	1.1	0.0 - 1.6 %		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.56	1.50 - 7.40 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Lymphocytes	2.22	0.90 - 3.10 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Monocytes	0.44	0.26 - 0.87 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.28	0.00 - 0.51 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Absolute Basophils	0.06	0.00 - 0.09 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.56	1.50 - 7.40 K/ mcL		01/31/2024 7:35 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:38 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

IRON +TIBC (IRON AND TIBC) - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Iron	34	50 - 170 mcg/dL		01/31/2024 12:29 PM PST	SCRIPPS LAB	
Transferrin	128	180 - 382 mg/ dL		01/31/2024 12:29 PM PST	SCRIPPS LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Iron Binding Capacity Calculated	160	250 - 450 mcg/dL		01/31/2024 12:29 PM PST	SCRIPPS LAB	
Iron Saturation	21	20 - 50 %		01/31/2024 12:29 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:38 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

VITAMIN B12 - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Vitamin B-12	718	213 - 816 pg/mL		01/31/2024 12:58 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:39 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

FOLATE - Final result (01/31/2024 6:08 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Folate	10.5	7.0 - 31.4 ng/mL		01/31/2024 12:58 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/31/2024 6:08 AM PST	01/31/2024 6:39 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

IR PICC LINE PLACEMENT - US/FLUROR GUIDANCE AS NEEDED (IR PICC LINE PLACEMENT - US/FLUORO GUIDANCE AS NEEDED) - Final result (01/30/2024 4:05 PM PST)

Anatomical Region	Laterality	Modality
		IR/OR Hybrid Labs

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/30/2024 4:11 PM PST	

Impressions

01/30/2024 4:11 PM PST
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance.

release

Narrative

01/30/2024 4:11 PM PST
EXAMINATION: IR PICC LINE PLACEMENT - US/FLUORO GUIDANCE AS NEEDED

DATE/TIME: 1/30/2024 4:05 PM

HISTORY: for long-term antibiotic administration

COMPARISON: None.

TECHNIQUE: A description of the procedure, including risks, complications and benefits of the procedure were explained to the patient who gave written and oral consent. The skin around the access site was prepped using maximal sterile barrier preparation technique which included caps, masks, sterile gowns, and sterile gloves worn by all personnel. In addition, the following was also utilized: hand hygiene, a large sterile sheet/drape, and 2% chlorhexidine for cutaneous antisepsis. In addition, sterile gel and a sterile transducer probe cover was utilized. Real-time ultrasound was used to evaluate potential access sites for venous access, and was used to directly visualize needle entry into the selected vein lumen. Using local anesthetic, the access vein was punctured with a 21G needle and an 0.018 inch guidewire was placed. A 5F sheath was placed and the PICC line was advanced into position using fluoroscopic guidance. The line was secured to the skin and a sterile dressing was
applied.

FLUOROSCOPY: Time 0.1-minute, One fluoroscopic spot image(s) obtained

FINDINGS:
VEIN ACCESSED: Right basilic vein.
CATHETER: 4 French Power PICC
CATHETER TIP POSITION: Distal superior vena cava.
MEDICATIONS: Local anesthetic.
IMAGING FINDINGS: No sonographic or fluoroscopic abnormalities.
OTHER: None.

Procedure Note

Trevor Nelson, MD - 01/30/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: IR PICC LINE PLACEMENT - US/FLUORO GUIDANCE AS NEEDED

DATE/TIME: 1/30/2024 4:05 PM

HISTORY: for long-term antibiotic administration

COMPARISON: None.

TECHNIQUE: A description of the procedure, including risks, complications and benefits of the procedure were explained to the patient who gave written and oral consent. The skin around the access site was prepped using maximal sterile barrier preparation technique which included caps, masks, sterile gowns, and sterile gloves worn by all personnel. In addition, the following was also utilized: hand hygiene, a large sterile sheet/drape, and 2% chlorhexidine for cutaneous antisepsis. In addition, sterile gel and a sterile transducer probe cover was utilized. Real-time ultrasound was used to evaluate potential access sites for venous access, and was used to directly visualize needle entry into the selected vein lumen. Using local anesthetic, the access vein was punctured with a 21G needle and an 0.018 inch guidewire was placed. A 5F sheath was placed and the PICC line was advanced into position using fluoroscopic guidance. The line was secured to the skin and a sterile dressing was
applied.

FLUOROSCOPY: Time 0.1-minute, One fluoroscopic spot image(s) obtained

FINDINGS:
VEIN ACCESSED: Right basilic vein.
CATHETER: 4 French Power PICC
CATHETER TIP POSITION: Distal superior vena cava.
MEDICATIONS: Local anesthetic.
IMAGING FINDINGS: No sonographic or fluoroscopic abnormalities.
OTHER: None.

IMPRESSION:
IMPRESSION: Successful upper extremity PICC line placement using sonographic and fluoroscopic guidance.

release

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	IMG IR PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/30/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.3	3.4 - 11.0 K/mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
RBC	2.33	3.98 - 5.25 M/mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
HGB	7.1	11.9 - 15.3 g/dL		01/30/2024 5:50 AM PST	MERCY LAB CV	
HCT	23.8	37.3 - 46.7 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/30/2024 5:50 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/30/2024 5:50 AM PST	MERCY LAB CV	
MCHC	29.8	31.0 - 36.0 g/dL		01/30/2024 5:50 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
RDW	13.4	11.7 - 14.9 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Platelet Count	496	150 - 425 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
MPV	8.0	9.0 - 12.8 fL		01/30/2024 5:50 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/30/2024 5:50 AM PST	MERCY LAB CV	
Neutrophils %	41.3	38.0 - 74.0 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
IG%	0.5	0.0 - 1.2 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Lymphocytes %	38.3	16.0 - 48.0 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Monocytes %	11.7	4.9 - 12.5 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Eosinophils %	7.0	0.0 - 9.5 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Basophils %	1.2	0.0 - 1.6 %		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Neutrophils	1.77	1.50 - 7.40 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.64	0.90 - 3.10 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Monocytes	0.50	0.26 - 0.87 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.30	0.00 - 0.51 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	
Preliminary Abs Neut	1.77	1.50 - 7.40 K/ mcL		01/30/2024 5:50 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/30/2024 5:21 AM PST	01/30/2024 5:43 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/30/2024 5:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/30/2024 6:00 AM PST	MERCY LAB CV	
Glucose	100	70 - 125 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Sodium	135	137 - 145 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Urea Nitrogen	27	7 - 17 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Creatinine	1.22	0.50 - 1.00 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	51	>60 mL/min/1.73m*2		01/30/2024 6:00 AM PST	MERCY LAB CV	
Anion Gap	-1	6 - 14 mmol/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Osmolality Calculated	285	280 - 305 mOsm/kg H20		01/30/2024 6:00 AM PST	MERCY LAB CV	
Calcium	8.1	8.4 - 10.2 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Protein, total	6.4	6.3 - 8.2 g/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	
Alkaline Phosphatase	75	38 - 126 Units/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
AST	23	14 - 36 Units/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
ALT	14	<=34 Units/L		01/30/2024 6:00 AM PST	MERCY LAB CV	
Bilirubin, Total	0.1	0.2 - 1.3 mg/dL		01/30/2024 6:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/30/2024 5:21 AM PST	01/30/2024 5:40 AM PST

Narrative

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MERCY LAB CV - 01/30/2024 6:00 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (01/29/2024 4:18 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	20.6	<10.0 mg/L		01/29/2024 4:58 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 4:18 PM PST	01/29/2024 4:38 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (01/29/2024 4:18 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	>140	<=30 mm/hr		01/29/2024 5:42 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 4:18 PM PST	01/29/2024 4:38 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Hai Shao MD, PhD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CT cervical spine with contrast (CT CERVICAL SPINE WITH CONTRAST) - Final result (01/29/2024 3:23 PM PST)

Anatomical Region	Laterality	Modality
C-spine		Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/29/2024 3:27 PM PST	

Impressions

01/29/2024 3:35 PM PST
IMPRESSION:

1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis.
2. Persistent prevertebral fluid, probably similar in extent to the recent MRI.

release

Narrative

01/29/2024 3:35 PM PST
EXAMINATION: CT CERVICAL SPINE WITH CONTRAST

DATE/TIME: 1/29/2024 3:23 PM

HISTORY: Prevertebral, retropharyngeal fluid collection

COMPARISON: MRI cervical spine 1/26/2024

TECHNIQUE: CT examination was performed with intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

There is reversal of cervical lordosis. There is C2-3 and C7-T1 mild anterolisthesis.

There are erosive changes of the C4-5 endplates. There is also irregularity of the C5-6 endplates.

There is persistent prevertebral fluid, extending from C2-C7.

There are multilevel degenerative changes, most pronounced at C6-7 where a disc osteophyte complex and uncovertebral hypertrophy cause mild canal stenosis and moderate left and mild right foraminal stenosis.

There is a partially visualized right pleural effusion.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 19; DLP: 317

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

Procedure Note

Benjamin Freeze, MD - 01/29/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: CT CERVICAL SPINE WITH CONTRAST

DATE/TIME: 1/29/2024 3:23 PM

HISTORY: Prevertebral, retropharyngeal fluid collection

COMPARISON: MRI cervical spine 1/26/2024

TECHNIQUE: CT examination was performed with intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

There is reversal of cervical lordosis. There is C2-3 and C7-T1 mild anterolisthesis.

There are erosive changes of the C4-5 endplates. There is also irregularity of the C5-6 endplates.

There is persistent prevertebral fluid, extending from C2-C7.

There are multilevel degenerative changes, most pronounced at C6-7 where a disc osteophyte complex and uncovertebral hypertrophy cause mild canal stenosis and moderate left and mild right foraminal stenosis.

There is a partially visualized right pleural effusion.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 19; DLP: 317

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

IMPRESSION:
IMPRESSION:

1. Findings consistent with C4-5 discitis-osteomyelitis, and probable C5-6 discitis-osteomyelitis.
2. Persistent prevertebral fluid, probably similar in extent to the recent MRI.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG CT PROCEDURES	Final Result

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (01/29/2024 2:59 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/29/2024 3:19 PM PST	

Impressions

01/29/2024 3:20 PM PST

Impressions

IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Narrative

01/29/2024 3:20 PM PST

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/29/2024 2:59 PM

HISTORY: bilateral leg edema

COMPARISON: 1/16/2024

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

Procedure Note

Paul Rickards, MD - 01/29/2024

Formatting of this note might be different from the original.

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/29/2024 2:59 PM

HISTORY: bilateral leg edema

COMPARISON: 1/16/2024

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

IMPRESSION:
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG US PROCEDURES	Final Result

X-ray chest 2 views with pa/ap and lateral (X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL) - Final result (01/29/2024 9:22 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/29/2024 9:26 AM PST	

Impressions

01/29/2024 9:26 AM PST
IMPRESSION: No significant change from prior.

release

Narrative

01/29/2024 9:26 AM PST
EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 1/29/2024 9:22 AM

HISTORY: f/u pleural fluid collection

COMPARISON: 1/25/2024

FINDINGS:

1. Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.
2. Clear left lung.
3. Stable cardiac silhouette size.

Procedure Note

Harold Pimenta, MD - 01/29/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY CHEST 2 VIEWS WITH PA/AP AND LATERAL

DATE/TIME: 1/29/2024 9:22 AM

HISTORY: f/u pleural fluid collection

COMPARISON: 1/25/2024

FINDINGS:

1. Relatively unchanged small to moderate right base pleural effusion with compressive changes of the right lower lobe.
2. Clear left lung.
3. Stable cardiac silhouette size.

IMPRESSION:
IMPRESSION: No significant change from prior.

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

C-REACTIVE PROTEIN - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	26.8	<10.0 mg/L		01/29/2024 10:22 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 6:07 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	>140	<=30 mm/hr		01/29/2024 11:20 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 7:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.6	3.4 - 11.0 K/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
RBC	2.48	3.98 - 5.25 M/mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
HGB	7.7	11.9 - 15.3 g/dL		01/29/2024 7:23 AM PST	MERCY LAB CV	
HCT	25.2	37.3 - 46.7 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/29/2024 7:23 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/29/2024 7:23 AM PST	MERCY LAB CV	
MCHC	30.6	31.0 - 36.0 g/dL		01/29/2024 7:23 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
RDW	13.2	11.7 - 14.9 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Platelet Count	552	150 - 425 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
MPV	8.4	9.0 - 12.8 fL		01/29/2024 7:23 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/29/2024 7:23 AM PST	MERCY LAB CV	
Neutrophils %	53.3	38.0 - 74.0 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
IG%	0.6	0.0 - 1.2 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Lymphocytes %	27.5	16.0 - 48.0 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Monocytes %	10.2	4.9 - 12.5 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Eosinophils %	7.1	0.0 - 9.5 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Basophils %	1.3	0.0 - 1.6 %		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.46	1.50 - 7.40 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.27	0.90 - 3.10 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Monocytes	0.47	0.26 - 0.87 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.33	0.00 - 0.51 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Absolute Basophils	0.06	0.00 - 0.09 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.46	1.50 - 7.40 K/ mcL		01/29/2024 7:23 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 7:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/29/2024 5:43 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/29/2024 6:52 AM PST	MERCY LAB CV	
Glucose	101	70 - 125 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Potassium	4.5	3.5 - 5.1 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Chloride	97	98 - 107 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Creatinine	0.75	0.50 - 1.00 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	92	>60 mL/min/1.73m*2		01/29/2024 6:52 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Osmolality Calculated	275	280 - 305 mOsm/kg H20		01/29/2024 6:52 AM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Protein, total	7.1	6.3 - 8.2 g/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	
Alkaline Phosphatase	86	38 - 126 Units/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
AST	25	14 - 36 Units/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/29/2024 6:52 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/29/2024 6:52 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/29/2024 5:43 AM PST	01/29/2024 6:07 AM PST

Narrative

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MERCY LAB CV - 01/29/2024 6:52 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

HIV ANTIGEN/ANTIBODY COMBO - Final result (01/28/2024 8:28 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HIV Antigen / Antibody Combo	Non Reactive	Non Reactive		01/28/2024 10:45 PM PST	SCRIPPS LAB	

Comment: HIV-1 p24 antigen and HIV-1/HIV-2 antibodies are not detected.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 8:28 PM PST	01/28/2024 8:31 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

CYCLIC CITRUL PEPTIDE AB, IGG (CYCLIC CITRUL PEPTIDE ANTIBODY, IGG) - Final result (01/28/2024 12:19 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cyclic Citrul Peptide Antibodies, IgG	1.0	<5.0 U/mL		01/28/2024 6:25 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 12:19 PM PST	01/28/2024 12:58 PM PST

Narrative

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SCRIPPS LAB - 01/28/2024 6:25 PM PST
Alinity i Anti-CCP results should not be used interchangeably with other manufacturers' methods for anti-CCP determinations.
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Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

RHEUMATOID FACTOR, QUANTITATIVE - Final result (01/28/2024 12:19 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Rheumatoid Factor	25	<30 U/mL		01/28/2024 5:53 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 12:19 PM PST	01/28/2024 12:58 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

ANA (ANTI-NUCLEAR ANTIBODY) - Final result (01/28/2024 12:19 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ANA	Negative	Negative		01/29/2024 11:07 AM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 12:19 PM PST	01/28/2024 12:58 PM PST

Narrative

SCRIPPS LAB - 01/29/2024 11:07 AM PST
Performed using Bio-Rad Autoimmune qualitative enzyme immunoassay (EIA).

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

GLUCOSE, POC - Final result (01/28/2024 7:25 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	130	70 - 125 mg/dL		01/28/2024 7:27 AM PST	MERCY LAB CV	
Operator ID	126,288			01/28/2024 7:27 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/28/2024 7:25 AM PST	01/28/2024 7:27 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

FERRITIN - Final result (01/28/2024 5:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Ferritin	113.52	4.63 - 204.00 ng/mL		01/28/2024 6:14 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 5:40 AM PST	01/28/2024 6:10 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

IRON +TIBC (IRON AND TIBC) - Final result (01/28/2024 5:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Iron	37	50 - 170 mcg/dL		01/28/2024 5:58 PM PST	SCRIPPS LAB	
Transferrin	123	180 - 382 mg/dL		01/28/2024 5:58 PM PST	SCRIPPS LAB	
Iron Binding Capacity Calculated	154	250 - 450 mcg/dL		01/28/2024 5:58 PM PST	SCRIPPS LAB	
Iron Saturation	24	20 - 50 %		01/28/2024 5:58 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 5:40 AM PST	01/28/2024 6:10 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

COMPREHENSIVE METABOLIC PANEL - Final result (01/28/2024 5:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/28/2024 7:00 AM PST	MERCY LAB CV	
Glucose	97	70 - 125 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Sodium	130	137 - 145 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Chloride	94	98 - 107 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Creatinine	0.67	0.50 - 1.00 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	101	>60 mL/min/1.73m*2		01/28/2024 7:00 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
Osmolality Calculated	270	280 - 305 mOsm/kg H20		01/28/2024 7:00 AM PST	MERCY LAB CV	
Calcium	8.4	8.4 - 10.2 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Protein, total	7.4	6.3 - 8.2 g/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Albumin	2.9	3.5 - 5.0 g/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	
Alkaline Phosphatase	81	38 - 126 Units/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
AST	32	14 - 36 Units/L		01/28/2024 7:00 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/28/2024 7:00 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/28/2024 7:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/28/2024 5:40 AM PST	01/28/2024 6:10 AM PST

Narrative

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MERCY LAB CV - 01/28/2024 7:00 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

HEMOGRAM - Final result (01/27/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.8	3.4 - 11.0 K/mcL		01/27/2024 7:03 AM PST	MERCY LAB CV	
RBC	2.42	3.98 - 5.25 M/mcL		01/27/2024 7:03 AM PST	MERCY LAB CV	
HGB	7.6	11.9 - 15.3 g/dL		01/27/2024 7:03 AM PST	MERCY LAB CV	
HCT	24.3	37.3 - 46.7 %		01/27/2024 7:03 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/27/2024 7:03 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/27/2024 7:03 AM PST	MERCY LAB CV	
MCHC	31.3	31.0 - 36.0 g/dL		01/27/2024 7:03 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/27/2024 7:03 AM PST	MERCY LAB CV	
Platelet Count	452	150 - 425 K/mcL		01/27/2024 7:03 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/27/2024 5:53 AM PST	01/27/2024 6:50 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/27/2024 5:53 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/27/2024 7:36 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Sodium	129	137 - 145 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Potassium	4.4	3.5 - 5.1 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Chloride	97	98 - 107 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Creatinine	0.63	0.50 - 1.00 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	102	>60 mL/min/1.73m*2		01/27/2024 7:36 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Osmolality Calculated	269	280 - 305 mOsm/kg H20		01/27/2024 7:36 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Protein, total	6.7	6.3 - 8.2 g/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	
Alkaline Phosphatase	71	38 - 126 Units/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
AST	26	14 - 36 Units/L		01/27/2024 7:36 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ALT	16	<=34 Units/L		01/27/2024 7:36 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/27/2024 7:36 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/27/2024 5:53 AM PST	01/27/2024 6:48 AM PST

Narrative

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MERCY LAB CV - 01/27/2024 7:36 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/26/2024 12:06 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	131	70 - 125 mg/dL		01/26/2024 12:09 PM PST	MERCY LAB CV	
Operator ID	255,308			01/26/2024 12:09 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/26/2024 12:06 PM PST	01/26/2024 12:09 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/26/2024 10:32 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/31/2024 12:09 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/26/2024 10:32 AM PST	01/26/2024 11:13 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/26/2024 10:23 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/31/2024 11:08 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of part of right upper limb / Unknown	Venipuncture / Unknown	01/26/2024 10:23 AM PST	01/26/2024 10:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MRI cervical spine with and without contrast (MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST) - Final result (01/26/2024 10:08 AM PST)

Anatomical Region	Laterality	Modality
C-spine		Magnetic Resonance

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/26/2024 10:14 AM PST	

Impressions

01/26/2024 10:23 AM PST
Impression:

Contrast imaging is very limited due to extensive motion.

Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema.

Findings are concerning for discitis/osteomyelitis at C4-C5.

release

Narrative

01/26/2024 10:23 AM PST
EXAMINATION: MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST

DATE/TIME: 1/26/2024 10:08 AM

HISTORY: concern for osteomyelitis from previous MRI without contrast

COMPARISON: Noncontrast MRI of the cervical spine performed 1 day prior.

TECHNIQUE: Multiplanar multisequence MRI examination was performed without and with contrast.

FINDINGS:

The exam is very limited due to extensive motion artifact.

Pre and postcontrast axial and postcontrast sagittal images were obtained.

There is a large amount of prevertebral edema with generalized enhancement.
This extends into the chest, anterior mediastinum, series 6 images 10 through 13. No discrete fluid collection.

There is also enhancement of the C4, C5 and C6 vertebral bodies with enhancement of the C4-C5 disc. The other discs are difficult to visualize due to severe disc height loss at C3-4, C5-6 and C6-7.

No definite abnormal enhancement within the spinal canal, epidural compartment..

Procedure Note

Anila Jonnala, MD - 01/26/2024
Formatting of this note might be different from the original.
EXAMINATION: MRI CERVICAL SPINE WITH AND WITHOUT CONTRAST

DATE/TIME: 1/26/2024 10:08 AM

HISTORY: concern for osteomyelitis from previous MRI without contrast

COMPARISON: Noncontrast MRI of the cervical spine performed 1 day prior.

TECHNIQUE: Multiplanar multisequence MRI examination was performed without and with contrast.

FINDINGS:

The exam is very limited due to extensive motion artifact.

Pre and postcontrast axial and postcontrast sagittal images were obtained.

There is a large amount of prevertebral edema with generalized enhancement.
This extends into the chest, anterior mediastinum, series 6 images 10 through 13. No discrete fluid collection.

There is also enhancement of the C4, C5 and C6 vertebral bodies with enhancement of the C4-C5 disc. The other discs are difficult to visualize due to severe disc height loss at C3-4, C5-6 and C6-7.

No definite abnormal enhancement within the spinal canal, epidural compartment..

IMPRESSION:
Impression:

Contrast imaging is very limited due to extensive motion.

Setting this aside, there is a large amount of partly enhancing posterior nasopharyngeal, prevertebral, anterior mediastinal fluid, and edema.

Findings are concerning for discitis/osteomyelitis at C4-C5.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG MRI PROCEDURES	Final Result

HEMOGRAM - Final result (01/26/2024 5:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.7	3.4 - 11.0 K/mcL		01/26/2024 7:56 AM PST	MERCY LAB CV	
RBC	2.67	3.98 - 5.25 M/mcL		01/26/2024 7:56 AM PST	MERCY LAB CV	
HGB	8.2	11.9 - 15.3 g/dL		01/26/2024 7:56 AM PST	MERCY LAB CV	
HCT	26.7	37.3 - 46.7 %		01/26/2024 7:56 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/26/2024 7:56 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/26/2024 7:56 AM PST	MERCY LAB CV	
MCHC	30.7	31.0 - 36.0 g/dL		01/26/2024 7:56 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/26/2024 7:56 AM PST	MERCY LAB CV	
Platelet Count	478	150 - 425 K/mcL		01/26/2024 7:56 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/26/2024 5:36 AM PST	01/26/2024 7:19 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/26/2024 5:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/26/2024 7:40 AM PST	MERCY LAB CV	
Glucose	100	70 - 125 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Potassium	4.5	3.5 - 5.1 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Chloride	98	98 - 107 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Creatinine	0.67	0.50 - 1.00 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	101	>60 mL/min/1.73m*2		01/26/2024 7:40 AM PST	MERCY LAB CV	
Anion Gap	1	6 - 14 mmol/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Osmolality Calculated	273	280 - 305 mOsm/kg H20		01/26/2024 7:40 AM PST	MERCY LAB CV	
Calcium	8.4	8.4 - 10.2 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Protein, total	6.9	6.3 - 8.2 g/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	
Alkaline Phosphatase	89	38 - 126 Units/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
AST	26	14 - 36 Units/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
ALT	14	<=34 Units/L		01/26/2024 7:40 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/26/2024 7:40 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/26/2024 5:36 AM PST	01/26/2024 7:20 AM PST

Narrative

MERCY LAB CV - 01/26/2024 7:40 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

C-REACTIVE PROTEIN - Final result (01/25/2024 10:52 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
C-Reactive Protein	32.7	<10.0 mg/L		01/25/2024 11:11 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 10:52 PM PST	01/25/2024 10:54 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SEDIMENTATION RATE, AUTOMATED (SEDIMENTATION RATE) - Final result (01/25/2024 10:52 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sed Rate	131	<=30 mm/hr		01/26/2024 12:38 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 10:52 PM PST	01/25/2024 10:54 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MRI cervical spine without contrast (MRI CERVICAL SPINE WITHOUT CONTRAST) - Final result (01/25/2024 6:33 PM PST)

Anatomical Region	Laterality	Modality
C-spine		Magnetic Resonance

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/25/2024 6:38 PM PST	

Impressions

01/25/2024 9:48 PM PST

IMPRESSION:
1. Abnormal edema at C4-C6 with prevertebral collection extending from C2-C7. This is very concerning for osteomyelitis with discitis at C4-C5 and infection spreading into the retropharyngeal, prevertebral soft tissues. If clinically indicated, consider postcontrast MRI of the cervical spine for evaluation.
2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.

Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024.

release 1419

A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

Narrative

01/25/2024 9:48 PM PST

EXAMINATION: MRI CERVICAL SPINE WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:33 PM

HISTORY: Evaluate severe ongoing debilitating neck pain. Cough, fever and neck pain.

COMPARISON: None.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI of the cervical spine was performed without contrast.

FINDINGS:

Abnormal edema at the vertebral bodies of C4, C5, C6 with indistinct anterior cortex at C4, C6.
Abnormal prevertebral collection, fluid from C2-C7. Fluid in the disc space of C4-C5.
No epidural collection.
Mild posterior disc bulge C4-T4 identified. There is moderate canal narrowing from disc bulge and short pedicles at C4-C7. Mild central canal narrowing C7-T1, T1-T2.
Moderate facet arthrosis identified. Axial images are motion degraded.

Severe right neural foramen narrowing C4-C5, moderate neural foramen narrowing bilaterally C5-C6, C6-C7.

Procedure Note

David Vu, MD - 01/25/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: MRI CERVICAL SPINE WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:33 PM

HISTORY: Evaluate severe ongoing debilitating neck pain. Cough, fever and neck pain.

COMPARISON: None.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI of the cervical spine was performed without contrast.

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Abnormal edema at the vertebral bodies of C4, C5, C6 with indistinct anterior cortex at C4, C6.
Abnormal prevertebral collection, fluid from C2-C7. Fluid in the disc space of C4-C5.
No epidural collection.
Mild posterior disc bulge C4-T4 identified. There is moderate canal narrowing from disc bulge and short pedicles at C4-C7. Mild central canal narrowing C7-T1, T1-T2.
Moderate facet arthrosis identified. Axial images are motion degraded.

Severe right neural foramen narrowing C4-C5, moderate neural foramen narrowing bilaterally C5-C6, C6-C7.

IMPRESSION:
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2. No definite epidural collection. Moderate central canal narrowing at C4-C5 and C5-6 from small disc bulges and congenitally short pedicles. Mild flattening of the cord at these levels.

Findings discussed with nurse Lauren Greenfield at 6:48 p.m. 1/25/2024.

release 1419

A Red 60mins actionable finding has been sent via the PowerConnect Actionable Findings system on 1/25/2024 6:42 PM, Message ID 364649. Receipt of this communication will be communicated to KHAYYAT, OMAR or responsible provider and will be documented in PowerConnect Actionable Finding System upon receiving acknowledgement.

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG MRI PROCEDURES	Final Result

MRI brain without contrast (MRI BRAIN WITHOUT CONTRAST) - Final result (01/25/2024 6:07 PM PST)

Anatomical Region	Laterality	Modality
Head		Magnetic Resonance

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/25/2024 6:10 PM PST	

Impressions

01/25/2024 9:48 PM PST
IMPRESSION:
1. No acute intracranial findings.
2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change.
3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process.

release

Narrative

01/25/2024 9:48 PM PST

Narrative

EXAMINATION: MRI BRAIN WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:07 PM

HISTORY: Severe occipital and neck headaches. Shortness of breath, pleural effusion.

COMPARISON: CT chest same day.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI examination was performed without contrast.

FINDINGS:
There is no acute infarct.

There is no intracranial hemorrhage. Punctate susceptibility at the left caudate body is probably from mineralization. Minimal FLAIR high signal in the white matter is likely from microvascular change. This is age-appropriate. The flow voids are intact.

Small amount of fluid in the right mastoid air cells. Mild mucosal thickening of the ethmoid air cells. The soft tissues are unremarkable.

Low signal within the diploic space, upper cervical spine from marrow replacing process or anemia.
Moderate degenerative change in the upper cervical spine at C3-C4, C4-C5. Moderate facet arthrosis in the upper cervical spine.
Moderate central canal narrowing C4-C5.

Procedure Note

David Vu, MD - 01/25/2024

Formatting of this note might be different from the original.

EXAMINATION: MRI BRAIN WITHOUT CONTRAST

DATE/TIME: 1/25/2024 6:07 PM

HISTORY: Severe occipital and neck headaches. Shortness of breath, pleural effusion.

COMPARISON: CT chest same day.

TECHNIQUE: Multiplanar multisequence 1.5 Tesla MRI examination was performed without contrast.

FINDINGS:
There is no acute infarct.

There is no intracranial hemorrhage. Punctate susceptibility at the left caudate body is probably from mineralization. Minimal FLAIR high signal in the white matter is likely from microvascular change. This is age-appropriate. The flow voids are intact.

Small amount of fluid in the right mastoid air cells. Mild mucosal thickening of the ethmoid air cells. The soft tissues are unremarkable.

Low signal within the diploic space, upper cervical spine from marrow replacing process or anemia.
Moderate degenerative change in the upper cervical spine at C3-C4, C4-C5. Moderate facet arthrosis in the upper cervical spine.
Moderate central canal narrowing C4-C5.

IMPRESSION:
IMPRESSION:
1. No acute intracranial findings.
2. Age-appropriate mild FLAIR high signal in the white matter likely from microvascular change.
3. Decreased T1 signal within the bony structures. This is nonspecific. This could be from hematopoietic marrow or from marrow replacing process.

release

Authorizing Provider	Result Type	Result Status
Omar Khayyat MD	IMG MRI PROCEDURES	Final Result

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/25/2024 1:14 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/25/2024 1:31 PM PST	

Impressions

01/25/2024 1:31 PM PST
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated

release

Narrative

01/25/2024 1:31 PM PST
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/25/2024 1:14 PM

HISTORY: status post Right thoracentesis

COMPARISON: 1/22/2024

FINDINGS:

LUNGS: Basilar right compressive change with volume loss
VASCULATURE: Normal. Unremarkable pulmonary vasculature.
CARDIAC: Normal. No cardiac silhouette abnormality.
MEDIASTINUM: Normal. No visible mass or adenopathy.
PLEURA: Ongoing moderate to large right effusion. No pneumothorax
BONES: Normal. No fracture or visible bony lesion.
OTHER: Negative.

Procedure Note

Trevor Nelson, MD - 01/25/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/25/2024 1:14 PM

HISTORY: status post Right thoracentesis

COMPARISON: 1/22/2024

FINDINGS:

LUNGS: Basilar right compressive change with volume loss
VASCULATURE: Normal. Unremarkable pulmonary vasculature.
CARDIAC: Normal. No cardiac silhouette abnormality.
MEDIASTINUM: Normal. No visible mass or adenopathy.
PLEURA: Ongoing moderate to large right effusion. No pneumothorax
BONES: Normal. No fracture or visible bony lesion.
OTHER: Negative.

IMPRESSION:
IMPRESSION: Similar exam post thoracentesis with no pneumothorax appreciated

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

PH, PLEURAL FLUID - Final result (01/25/2024 10:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Pleural Fluid	7.27	Not Established		01/25/2024 10:51 AM PST	MERCY LAB CV	
Operator ID	202,545			01/25/2024 10:51 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:35 AM PST	01/25/2024 10:43 AM PST

Narrative

MERCY LAB CV - 01/25/2024 10:51 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PATHOLOGIST FLUID COMMENT - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Pathologist Review, CSF/ Body Fluids	Morphologic analysis of the smear shows severe acute inflammation with PMN vacuoles without atypia. Carla Stayboldt, M.D.			01/25/2024 4:18 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, WOUND (WOUND/FLUID/TISSUE CULTURE W/STAIN) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Wound Culture	No Growth Final		MIC	01/29/2024 7:46 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	Many WBCs			01/29/2024 7:46 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	No organisms seen			01/29/2024 7:46 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

ALBUMIN BODY FLUID (ALBUMIN, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Albumin (FL)	1.7	Not Established g/dL		01/25/2024 12:25 PM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/25/2024 12:25 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 12:25 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE BODY FLUID (GLUCOSE, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/25/2024 12:25 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose (FL)	65	Not Established mg/dL		01/25/2024 12:25 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 12:25 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTATE DEHYDROGENASE BODY FLUID (LACTATE DEHYDROGENASE, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/25/2024 1:52 PM PST	MERCY LAB CV	
Lactate Dehrdrogenase (FL)	1,862	Not Established Units/L		01/25/2024 1:52 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 1:52 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTEIN BODY FLUID (PROTEIN, BODY FLUID) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/25/2024 12:25 PM PST	MERCY LAB CV	
Protein (FL)	4.3	Not Established g/dL		01/25/2024 12:25 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 12:25 PM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BODY FLUID CELL COUNT WITH DIFF (BODY FLUID CELL COUNT WITH DIFFERENTIAL) - Final result (01/25/2024 10:34 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Volume (ML)	265.0	mL		01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Color	Orange			01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Appearance	Slightly Hazy			01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Nucleated Cell Count	6,055	/mcL		01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid RBC Count	17,000	/mcL		01/25/2024 2:42 PM PST	MERCY LAB CV	
Fluid Neutrophils %	81	%		01/25/2024 2:42 PM PST	MERCY LAB CV	
Comment: Appended report. These results have been appended to a previously preliminary verified report.						
Fluid Lymphocytes %	10	%		01/25/2024 2:42 PM PST	MERCY LAB CV	
Comment: Appended report. These results have been appended to a previously preliminary verified report.						
Fluid Mononuclears %	9	%		01/25/2024 2:42 PM PST	MERCY LAB CV	
Comment: Appended report. These results have been appended to a previously preliminary verified report.						

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/25/2024 10:34 AM PST	01/25/2024 10:44 AM PST

Narrative

MERCY LAB CV - 01/25/2024 2:42 PM PST
Reference Interval has not been established for this specimen type.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

HEMOGRAM - Final result (01/25/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.6	3.4 - 11.0 K/mcL		01/25/2024 6:26 AM PST	MERCY LAB CV	
RBC	2.54	3.98 - 5.25 M/mcL		01/25/2024 6:26 AM PST	MERCY LAB CV	
HGB	7.8	11.9 - 15.3 g/dL		01/25/2024 6:26 AM PST	MERCY LAB CV	
HCT	25.4	37.3 - 46.7 %		01/25/2024 6:26 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/25/2024 6:26 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/25/2024 6:26 AM PST	MERCY LAB CV	
MCHC	30.7	31.0 - 36.0 g/dL		01/25/2024 6:26 AM PST	MERCY LAB CV	
RDW	13.1	11.7 - 14.9 %		01/25/2024 6:26 AM PST	MERCY LAB CV	
Platelet Count	413	150 - 425 K/mcL		01/25/2024 6:26 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 5:31 AM PST	01/25/2024 6:01 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTATE DEHYDROGENASE (LACTATE DEHYDROGENASE, TOTAL) - Final result (01/25/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Lactate Dehydrogenase	141	120 - 246 Units/L		01/25/2024 6:27 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 5:31 AM PST	01/25/2024 5:59 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/25/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/25/2024 6:28 AM PST	MERCY LAB CV	
Glucose	120	70 - 125 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Chloride	97	98 - 107 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/25/2024 6:28 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Osmolality Calculated	276	280 - 305 mOsm/kg H20		01/25/2024 6:28 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Protein, total	6.7	6.3 - 8.2 g/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	
Alkaline Phosphatase	74	38 - 126 Units/L		01/25/2024 6:28 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
AST	27	14 - 36 Units/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		01/25/2024 6:28 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/25/2024 6:28 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/25/2024 5:31 AM PST	01/25/2024 5:59 AM PST

Narrative

MERCY LAB CV - 01/25/2024 6:28 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/24/2024 5:48 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.8	3.4 - 11.0 K/mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
RBC	2.58	3.98 - 5.25 M/mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
HGB	8.0	11.9 - 15.3 g/dL		01/24/2024 6:30 AM PST	MERCY LAB CV	
HCT	26.3	37.3 - 46.7 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/24/2024 6:30 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/24/2024 6:30 AM PST	MERCY LAB CV	
MCHC	30.4	31.0 - 36.0 g/dL		01/24/2024 6:30 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/24/2024 6:30 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	364	150 - 425 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
MPV	8.6	9.0 - 12.8 fL		01/24/2024 6:30 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/24/2024 6:30 AM PST	MERCY LAB CV	
Neutrophils %	65.7	38.0 - 74.0 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
IG%	0.4	0.0 - 1.2 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Lymphocytes %	19.3	16.0 - 48.0 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Monocytes %	11.1	4.9 - 12.5 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Eosinophils %	2.7	0.0 - 9.5 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Basophils %	0.8	0.0 - 1.6 %		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.13	1.50 - 7.40 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Lymphocytes	0.92	0.90 - 3.10 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Monocytes	0.53	0.26 - 0.87 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.13	0.00 - 0.51 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	
Preliminary Abs Neut	3.13	1.50 - 7.40 K/ mcL		01/24/2024 6:30 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/24/2024 5:48 AM PST	01/24/2024 6:21 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/24/2024 5:48 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/24/2024 6:58 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Potassium	4.9	3.5 - 5.1 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Chloride	98	98 - 107 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/24/2024 6:58 AM PST	MERCY LAB CV	
Anion Gap	-3	6 - 14 mmol/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Osmolality Calculated	275	280 - 305 mOsm/kg H20		01/24/2024 6:58 AM PST	MERCY LAB CV	
Calcium	8.0	8.4 - 10.2 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Protein, total	6.6	6.3 - 8.2 g/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	
Alkaline Phosphatase	74	38 - 126 Units/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
AST	25	14 - 36 Units/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
ALT	17	<=34 Units/L		01/24/2024 6:58 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/24/2024 6:58 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/24/2024 5:48 AM PST	01/24/2024 6:23 AM PST

Narrative

MERCY LAB CV - 01/24/2024 6:58 AM PST

```
for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/23/2024 5:44 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.2	3.4 - 11.0 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
RBC	2.52	3.98 - 5.25 M/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
HGB	7.8	11.9 - 15.3 g/dL		01/23/2024 6:58 AM PST	MERCY LAB CV	
HCT	25.6	37.3 - 46.7 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/23/2024 6:58 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/23/2024 6:58 AM PST	MERCY LAB CV	
MCHC	30.5	31.0 - 36.0 g/dL		01/23/2024 6:58 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Platelet Count	353	150 - 425 K/mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
MPV	8.8	9.0 - 12.8 fL		01/23/2024 6:58 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/23/2024 6:58 AM PST	MERCY LAB CV	
Neutrophils %	57.0	38.0 - 74.0 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
IG%	0.7	0.0 - 1.2 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Lymphocytes %	25.2	16.0 - 48.0 %		01/23/2024 6:58 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	14.2	4.9 - 12.5 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Eosinophils %	2.2	0.0 - 9.5 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Neutrophils	2.37	1.50 - 7.40 K/ mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/ mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.05	0.90 - 3.10 K/ mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Monocytes	0.59	0.26 - 0.87 K/ mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.09	0.00 - 0.51 K/ mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/ mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	
Preliminary Abs Neut	2.37	1.50 - 7.40 K/ mcL		01/23/2024 6:58 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/23/2024 5:44 AM PST	01/23/2024 6:20 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/23/2024 5:44 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/23/2024 6:46 AM PST	MERCY LAB CV	
Glucose	92	70 - 125 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Sodium	128	137 - 145 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Potassium	4.1	3.5 - 5.1 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	96	98 - 107 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Urea Nitrogen	16	7 - 17 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Creatinine	0.80	0.50 - 1.00 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	85	>60 mL/min/1.73m*2		01/23/2024 6:46 AM PST	MERCY LAB CV	
Anion Gap	-5	6 - 14 mmol/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Osmolality Calculated	267	280 - 305 mOsm/kg H20		01/23/2024 6:46 AM PST	MERCY LAB CV	
Calcium	7.8	8.4 - 10.2 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Protein, total	6.3	6.3 - 8.2 g/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Albumin	2.5	3.5 - 5.0 g/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	
Alkaline Phosphatase	68	38 - 126 Units/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
AST	25	14 - 36 Units/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
ALT	14	<=34 Units/L		01/23/2024 6:46 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/23/2024 6:46 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/23/2024 5:44 AM PST	01/23/2024 6:20 AM PST

Narrative

MERCY LAB CV - 01/23/2024 6:46 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/22/2024 12:09 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/22/2024 12:25 PM PST	

Impressions

01/22/2024 12:26 PM PST
IMPRESSION:
1. Right chest tube removal with no pneumothorax. There is stable pleural-parenchymal opacity at the right lung base.

2. Mild left basilar atelectatic versus low-grade inflammatory change.

release
CC Dr: p: f:

Narrative

01/22/2024 12:26 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/22/2024 12:09 PM

HISTORY: dyspnea

COMPARISON: 1/21/2024

FINDINGS: Interval removal of the right chest tube. There is continued moderate pleural thickening at the right lung base with associated mild to moderate patchy parenchymal opacity and streak-like change. Minimal streak-like changes are seen at the left lung base with minimal blunting of left lateral costophrenic angle.

Stable mild cardiomegaly. No pneumothorax or pulmonary venous hypertension.

Procedure Note

Ross Christensen, MD - 01/22/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/22/2024 12:09 PM

HISTORY: dyspnea

COMPARISON: 1/21/2024

FINDINGS: Interval removal of the right chest tube. There is continued moderate pleural thickening at the right lung base with associated mild to moderate patchy parenchymal opacity and streak-like change. Minimal streak-like changes are seen at the left lung base with minimal blunting of left lateral costophrenic angle.

Stable mild cardiomegaly. No pneumothorax or pulmonary venous hypertension.

IMPRESSION:
IMPRESSION:
1. Right chest tube removal with no pneumothorax. There is stable pleural-parenchymal opacity at the right lung base.

2. Mild left basilar atelectatic versus low-grade inflammatory change.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Diep Dao DO	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/22/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.8	3.4 - 11.0 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
RBC	2.69	3.98 - 5.25 M/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
HGB	8.4	11.9 - 15.3 g/dL		01/22/2024 6:44 AM PST	MERCY LAB CV	
HCT	26.8	37.3 - 46.7 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/22/2024 6:44 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/22/2024 6:44 AM PST	MERCY LAB CV	
MCHC	31.3	31.0 - 36.0 g/dL		01/22/2024 6:44 AM PST	MERCY LAB CV	
RDW	13.3	11.7 - 14.9 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Platelet Count	368	150 - 425 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MPV	8.9	9.0 - 12.8 fL		01/22/2024 6:44 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/22/2024 6:44 AM PST	MERCY LAB CV	
Neutrophils %	67.6	38.0 - 74.0 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
IG%	0.4	0.0 - 1.2 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Lymphocytes %	17.6	16.0 - 48.0 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Monocytes %	12.2	4.9 - 12.5 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Eosinophils %	1.2	0.0 - 9.5 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Basophils %	1.0	0.0 - 1.6 %		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.27	1.50 - 7.40 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
IG#	0.02	0.00 - 0.10 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Lymphocytes	0.85	0.90 - 3.10 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Monocytes	0.59	0.26 - 0.87 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.06	0.00 - 0.51 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	
Preliminary Abs Neut	3.27	1.50 - 7.40 K/mcL		01/22/2024 6:44 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/22/2024 5:31 AM PST	01/22/2024 5:56 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/22/2024 5:31 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/22/2024 7:13 AM PST	MERCY LAB CV	
Glucose	105	70 - 125 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
CO2, Total	35	22 - 30 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Urea Nitrogen	15	7 - 17 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Creatinine	0.86	0.50 - 1.00 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	78	>60 mL/min/1.73m*2		01/22/2024 7:13 AM PST	MERCY LAB CV	
Anion Gap	0	6 - 14 mmol/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Osmolality Calculated	273	280 - 305 mOsm/kg H20		01/22/2024 7:13 AM PST	MERCY LAB CV	
Calcium	7.9	8.4 - 10.2 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Protein, total	5.9	6.3 - 8.2 g/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Albumin	2.4	3.5 - 5.0 g/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	
Alkaline Phosphatase	61	38 - 126 Units/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
AST	23	14 - 36 Units/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
ALT	11	<=34 Units/L		01/22/2024 7:13 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/22/2024 7:13 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/22/2024 5:31 AM PST	01/22/2024 5:56 AM PST

Narrative

MERCY LAB CV - 01/22/2024 7:13 AM PST

Narrative

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for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

OSMOLALITY - Final result (01/21/2024 3:51 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality	281	275 - 295 mOsm/kg H2O		01/21/2024 7:57 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 3:51 PM PST	01/21/2024 3:55 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Jennifer Pham MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SODIUM - Final result (01/21/2024 3:51 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	126	137 - 145 mmol/L		01/21/2024 4:07 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 3:51 PM PST	01/21/2024 3:55 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/21/2024 5:43 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/21/2024 9:32 AM PST	

Impressions

01/21/2024 9:34 AM PST

IMPRESSION:

Right pleural catheter in place.

Small to moderate pleural effusion and patchy right basilar opacity, probably atelectasis.

No pneumothorax.

release

Narrative

01/21/2024 9:34 AM PST

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/21/2024 5:43 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/20/2024.

FINDINGS:

There is a right pleural catheter in place. A small to moderate right pleural effusion is not significantly changed. There is no pneumothorax. Patchy right basilar opacity is noted.

The left lung field is clear. There is a trace left pleural effusion.

The cardiac silhouette is magnified.

Procedure Note

Tomer Roth, MD - 01/21/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/21/2024 5:43 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/20/2024.

FINDINGS:

There is a right pleural catheter in place. A small to moderate right pleural effusion is not significantly changed. There is no pneumothorax. Patchy right basilar opacity is noted.

The left lung field is clear. There is a trace left pleural effusion.

The cardiac silhouette is magnified.

IMPRESSION:
IMPRESSION:

Right pleural catheter in place.

Small to moderate pleural effusion and patchy right basilar opacity, probably atelectasis.

No pneumothorax.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CYCLIC CITRUL PEPTIDE AB, IGG (CYCLIC CITRUL PEPTIDE ANTIBODY, IGG) - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cyclic Citrul Peptide Antibodies, IgG	0.7	<5.0 U/mL		01/21/2024 6:17 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:45 AM PST

Narrative

SCRIPPS LAB - 01/21/2024 6:17 PM PST
Alinity i Anti-CCP results should not be used interchangeably with other manufacturers' methods for anti-CCP determinations.

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

RHEUMATOID FACTOR, QUANTITATIVE - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Rheumatoid Factor	19	<30 U/mL		01/21/2024 6:28 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:47 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

ANA (ANTI-NUCLEAR ANTIBODY) - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ANA	Negative	Negative		01/22/2024 11:18 AM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:47 AM PST

Narrative

SCRIPPS LAB - 01/22/2024 11:18 AM PST
Performed using Bio-Rad Autoimmune qualitative enzyme immunoassay (EIA).

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

CBC WITH DIFFERENTIAL - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	5.7	3.4 - 11.0 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
RBC	2.74	3.98 - 5.25 M/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
HGB	8.6	11.9 - 15.3 g/dL		01/21/2024 6:00 AM PST	MERCY LAB CV	
HCT	27.4	37.3 - 46.7 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		01/21/2024 6:00 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCH	31	26 - 33 pg		01/21/2024 6:00 AM PST	MERCY LAB CV	
MCHC	31.4	31.0 - 36.0 g/dL		01/21/2024 6:00 AM PST	MERCY LAB CV	
RDW	13.2	11.7 - 14.9 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Platelet Count	404	150 - 425 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
MPV	8.9	9.0 - 12.8 fL		01/21/2024 6:00 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/21/2024 6:00 AM PST	MERCY LAB CV	
Neutrophils %	67.4	38.0 - 74.0 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
IG%	0.7	0.0 - 1.2 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Lymphocytes %	19.9	16.0 - 48.0 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Monocytes %	10.0	4.9 - 12.5 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Eosinophils %	1.1	0.0 - 9.5 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Basophils %	0.9	0.0 - 1.6 %		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Neutrophils	3.83	1.50 - 7.40 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
IG#	0.04	0.00 - 0.10 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.13	0.90 - 3.10 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Monocytes	0.57	0.26 - 0.87 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.06	0.00 - 0.51 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	
Preliminary Abs Neut	3.83	1.50 - 7.40 K/mcL		01/21/2024 6:00 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:44 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/21/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/21/2024 6:08 AM PST	MERCY LAB CV	
Glucose	129	70 - 125 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Sodium	127	137 - 145 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Urea Nitrogen	10	7 - 17 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/21/2024 6:08 AM PST	MERCY LAB CV	
Anion Gap	-5	6 - 14 mmol/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Osmolality Calculated	265	280 - 305 mOsm/kg H20		01/21/2024 6:08 AM PST	MERCY LAB CV	
Calcium	8.1	8.4 - 10.2 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Protein, total	6.0	6.3 - 8.2 g/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Albumin	2.4	3.5 - 5.0 g/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	
Alkaline Phosphatase	57	38 - 126 Units/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
AST	21	14 - 36 Units/L		01/21/2024 6:08 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ALT	16	<=34 Units/L		01/21/2024 6:08 AM PST	MERCY LAB CV	
Bilirubin, Total	0.2	0.2 - 1.3 mg/dL		01/21/2024 6:08 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/21/2024 5:28 AM PST	01/21/2024 5:45 AM PST

Narrative

```
MERCY LAB CV - 01/21/2024 6:08 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

OSMOLALITY,URINE (OSMOLALITY, URINE) - Final result (01/20/2024 3:47 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality (UR)	152	50 - 1,200 mOsm/kg H2O		01/20/2024 4:43 PM PST	MERCY LAB CV	

Comment: Note: New Reference Range

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/20/2024 3:47 PM PST	01/20/2024 4:24 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CREATININE, RANDOM URINE - Final result (01/20/2024 3:46 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Creatinine, (UR)	15.62	Not Established mg/dL		01/20/2024 4:38 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Voided urine specimen / Unknown		01/20/2024 3:46 PM PST	01/20/2024 4:24 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

SODIUM, RANDOM URINE - Final result (01/20/2024 3:46 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium, (Ur)	17	Not Established mmol/L		01/20/2024 4:38 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Voided urine specimen / Unknown		01/20/2024 3:46 PM PST	01/20/2024 4:24 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/20/2024 6:48 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/20/2024 8:06 AM PST	

Impressions

01/20/2024 8:07 AM PST
IMPRESSION: Right-sided pigtail chest tube remains in place. There is minimal basilar volume loss. Cardiomegaly again noted. Mild redistribution.

release
CC Dr: p: f:

Narrative

01/20/2024 8:07 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 6:48 AM

HISTORY: Respiratory failure.

COMPARISON: 0112 hours.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

Procedure Note

Richard Abello, MD - 01/20/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 6:48 AM

HISTORY: Respiratory failure.

COMPARISON: 0112 hours.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

IMPRESSION:
IMPRESSION: Right-sided pigtail chest tube remains in place. There is minimal basilar volume loss. Cardiomegaly again noted. Mild redistribution.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/20/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	7.6	3.4 - 11.0 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
RBC	3.12	3.98 - 5.25 M/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
HGB	9.8	11.9 - 15.3 g/dL		01/20/2024 6:07 AM PST	MERCY LAB CV	
HCT	31.6	37.3 - 46.7 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		01/20/2024 6:07 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/20/2024 6:07 AM PST	MERCY LAB CV	
MCHC	31.0	31.0 - 36.0 g/dL		01/20/2024 6:07 AM PST	MERCY LAB CV	
RDW	13.6	11.7 - 14.9 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Platelet Count	466	150 - 425 K/mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
MPV	9.0	9.0 - 12.8 fL		01/20/2024 6:07 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/20/2024 6:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Neutrophils %	73.8	38.0 - 74.0 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Lymphocytes %	15.5	16.0 - 48.0 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Monocytes %	8.7	4.9 - 12.5 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Eosinophils %	0.5	0.0 - 9.5 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Basophils %	0.7	0.0 - 1.6 %		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	5.61	1.50 - 7.40 K/ mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
IG#	0.06	0.00 - 0.10 K/ mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.18	0.90 - 3.10 K/ mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.66	0.26 - 0.87 K/ mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.04	0.00 - 0.51 K/ mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/ mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	5.61	1.50 - 7.40 K/ mcL		01/20/2024 6:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/20/2024 5:35 AM PST	01/20/2024 6:01 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/20/2024 5:35 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/20/2024 6:42 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	96	70 - 125 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Sodium	129	137 - 145 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Potassium	4.9	3.5 - 5.1 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Chloride	96	98 - 107 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
CO2, Total	35	22 - 30 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Urea Nitrogen	12	7 - 17 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Creatinine	0.66	0.50 - 1.00 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	101	>60 mL/min/1.73m*2		01/20/2024 6:42 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Osmolality Calculated	268	280 - 305 mOsm/kg H20		01/20/2024 6:42 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Protein, total	6.2	6.3 - 8.2 g/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Albumin	2.5	3.5 - 5.0 g/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	
Alkaline Phosphatase	64	38 - 126 Units/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
AST	18	14 - 36 Units/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
ALT	12	<=34 Units/L		01/20/2024 6:42 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/20/2024 6:42 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/20/2024 5:35 AM PST	01/20/2024 6:01 AM PST

Narrative

MERCY LAB CV - 01/20/2024 6:42 AM PST

Narrative

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/20/2024 1:22 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/20/2024 8:50 AM PST	

Impressions

01/20/2024 8:51 AM PST
IMPRESSION: Stable position of right chest tube, some loculated pleural fluid and no pneumothorax. Mild bibasilar volume loss.

These findings are concordant with the preliminary interpretation provided by StatRad radiologist,

Statrad Code: A

release
CC Dr: p: f:

Narrative

01/20/2024 8:51 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 1:22 AM

HISTORY: Pain/cough/SOB/infiltrate.

COMPARISON: Prior day.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

Procedure Note

Richard Abello, MD - 01/20/2024

Procedure Note

Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/20/2024 1:22 AM

HISTORY: Pain/cough/SOB/infiltrate.

COMPARISON: Prior day.

FINDINGS: The lung appearance is stable. No difference in cardiac size.

IMPRESSION:
IMPRESSION: Stable position of right chest tube, some loculated pleural fluid and no pneumothorax. Mild bibasilar volume loss.

These findings are concordant with the preliminary interpretation provided by StatRad radiologist,

Statrad Code: A

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Jose Pena MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

X-ray foot right 3 or more views (X-RAY FOOT RIGHT 3 OR MORE VIEWS) - Final result (01/19/2024 3:51 PM PST)

Anatomical Region	Laterality	Modality
Lower Extremities, Foot	Right	Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/19/2024 4:19 PM PST	

Impressions

01/19/2024 4:20 PM PST
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

release

Narrative

01/19/2024 4:20 PM PST
EXAMINATION: X-RAY FOOT RIGHT 3 OR MORE VIEWS

DATE/TIME: 1/19/2024 3:51 PM

HISTORY: pain and swelling.

COMPARISON: None.

FINDINGS:

Chronic erosions involve the distal first metatarsal with sclerotic margins. No acute pathology. No evidence of fracture. No lytic changes.

Hallux valgus.

Procedure Note

Brian Shore, MD - 01/19/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY FOOT RIGHT 3 OR MORE VIEWS

DATE/TIME: 1/19/2024 3:51 PM

HISTORY: pain and swelling.

COMPARISON: None.

FINDINGS:

Chronic erosions involve the distal first metatarsal with sclerotic margins. No acute pathology. No evidence of fracture. No lytic changes.

Hallux valgus.

IMPRESSION:
IMPRESSION:
1. Chronic erosions of the distal first metatarsal likely related to gout
2.. No acute bony pathology.

release

Authorizing Provider	Result Type	Result Status
Diep Dao DO	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/19/2024 2:25 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/19/2024 2:45 PM PST	

Impressions

01/19/2024 2:46 PM PST
IMPRESSION: No change.

release

Narrative

01/19/2024 2:46 PM PST
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 2:25 PM

HISTORY: Respiratory failure

COMPARISON: 1/19/2024, earlier time.

FINDINGS:

1. Stable position of smallbore right chest tube. No discernible pneumothorax.
2. Persistent small peripheral right pleural effusion.
3. Unchanged haziness in the right lung base.
4. The left lung is clear. Stable cardiomediastinal silhouette.

Procedure Note

Harold Pimenta, MD - 01/19/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 2:25 PM

HISTORY: Respiratory failure

COMPARISON: 1/19/2024, earlier time.

FINDINGS:

1. Stable position of smallbore right chest tube. No discernible pneumothorax.
2. Persistent small peripheral right pleural effusion.
3. Unchanged haziness in the right lung base.
4. The left lung is clear. Stable cardiomediastinal silhouette.

IMPRESSION:
IMPRESSION: No change.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

SURVEILLANCE MRSA CULTURE - Final result (01/19/2024 12:30 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Staph Screen culture	NO Methicillin/ Oxacillin Resistant Staphylococcus aureus		MIC	01/20/2024 2:12 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasal structure / Unknown		01/19/2024 12:30 PM PST	01/19/2024 12:55 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/19/2024 6:38 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/19/2024 7:51 AM PST	

Impressions

01/19/2024 8:17 AM PST

IMPRESSION: No significant change in a right-sided chest tube, a moderate right pleural effusion, right lower lung field infiltrates.

release

Narrative

01/19/2024 8:17 AM PST

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 6:38 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: Chest x-ray 1/18/2024, CT chest 1/16/2024

FINDINGS:
Right-sided chest tube remains with its tip unchanged within the peripheral aspect of the right midlung field.
No significant change in the extent of a moderate right pleural effusion, patchy infiltrate within the right middle lobe and right lower lobe.

The left lung is clear.

Stable cardiomegaly and mediastinal contour.

Procedure Note

Anila Jonnala, MD - 01/19/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/19/2024 6:38 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: Chest x-ray 1/18/2024, CT chest 1/16/2024

FINDINGS:
Right-sided chest tube remains with its tip unchanged within the peripheral aspect of the right midlung field.
No significant change in the extent of a moderate right pleural effusion, patchy infiltrate within the right middle lobe and right lower lobe.

The left lung is clear.

Stable cardiomegaly and mediastinal contour.

IMPRESSION:
IMPRESSION: No significant change in a right-sided chest tube, a moderate right pleural effusion, right lower lung field infiltrates.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

URIC ACID - Final result (01/19/2024 4:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Uric Acid	2.6	2.5 - 6.2 mg/dL		01/19/2024 12:58 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/19/2024 4:22 AM PST	01/19/2024 5:33 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Diep Dao DO	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/19/2024 4:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	9.6	3.4 - 11.0 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
RBC	3.21	3.98 - 5.25 M/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
HGB	10.2	11.9 - 15.3 g/dL		01/19/2024 7:09 AM PST	MERCY LAB CV	
HCT	32.8	37.3 - 46.7 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/19/2024 7:09 AM PST	MERCY LAB CV	
MCH	32	26 - 33 pg		01/19/2024 7:09 AM PST	MERCY LAB CV	
MCHC	31.1	31.0 - 36.0 g/dL		01/19/2024 7:09 AM PST	MERCY LAB CV	
RDW	13.7	11.7 - 14.9 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Platelet Count	426	150 - 425 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
MPV				01/19/2024 7:09 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/19/2024 7:09 AM PST	MERCY LAB CV	
Neutrophils %	78.9	38.0 - 74.0 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
IG%	0.9	0.0 - 1.2 %		01/19/2024 7:09 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Lymphocytes %	11.4	16.0 - 48.0 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Monocytes %	7.6	4.9 - 12.5 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Eosinophils %	0.7	0.0 - 9.5 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Basophils %	0.5	0.0 - 1.6 %		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Neutrophils	7.56	1.50 - 7.40 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
IG#	0.09	0.00 - 0.10 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.09	0.90 - 3.10 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Monocytes	0.73	0.26 - 0.87 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.07	0.00 - 0.51 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	
Preliminary Abs Neut	7.56	1.50 - 7.40 K/mcL		01/19/2024 7:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/19/2024 4:22 AM PST	01/19/2024 5:33 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/19/2024 4:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/19/2024 6:14 AM PST	MERCY LAB CV	
Glucose	94	70 - 125 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Sodium	130	137 - 145 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Potassium	4.7	3.5 - 5.1 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Creatinine	0.60	0.50 - 1.00 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	103	>60 mL/min/1.73m*2		01/19/2024 6:14 AM PST	MERCY LAB CV	
Anion Gap	-2	6 - 14 mmol/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Osmolality Calculated	270	280 - 305 mOsm/kg H20		01/19/2024 6:14 AM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Protein, total	6.2	6.3 - 8.2 g/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	
Alkaline Phosphatase	62	38 - 126 Units/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
AST	24	14 - 36 Units/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
ALT	13	<=34 Units/L		01/19/2024 6:14 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/19/2024 6:14 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/19/2024 4:22 AM PST	01/19/2024 5:33 AM PST

Narrative

MERCY LAB CV - 01/19/2024 6:14 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/18/2024 9:38 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/18/2024 9:39 AM PST	

Impressions

01/18/2024 9:40 AM PST
IMPRESSION: Stable pleural-parenchymal opacity the right lung base with no pneumothorax.

release
CC Dr: p: f:

Narrative

01/18/2024 9:40 AM PST
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/18/2024 9:38 AM

HISTORY: empyema

COMPARISON: 1/17/2024

FINDINGS: Stable position of the pigtail right chest tube overlying the periphery the right midlung zone. There is ongoing mild right lateral pleural thickening with blunting of the right lateral costophrenic angle, and continued mild hazy and streak-like opacity at the right lung base heart size is at the upper limits of normal with no new mediastinal or hilar abnormality. No pneumothorax.

Procedure Note

Ross Christensen, MD - 01/18/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/18/2024 9:38 AM

HISTORY: empyema

COMPARISON: 1/17/2024

FINDINGS: Stable position of the pigtail right chest tube overlying the periphery the right midlung zone. There is ongoing mild right lateral pleural thickening with blunting of the right lateral costophrenic angle, and continued mild hazy and streak-like opacity at the right lung base heart size is at the upper limits of normal with no new mediastinal or hilar abnormality. No pneumothorax.

IMPRESSION:
IMPRESSION: Stable pleural-parenchymal opacity the right lung base with no pneumothorax.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/18/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	11.6	3.4 - 11.0 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
RBC	3.29	3.98 - 5.25 M/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
HGB	10.1	11.9 - 15.3 g/dL		01/18/2024 7:25 AM PST	MERCY LAB CV	
HCT	34.2	37.3 - 46.7 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
MCV	104	81 - 100 fL		01/18/2024 7:25 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/18/2024 7:25 AM PST	MERCY LAB CV	
MCHC	29.5	31.0 - 36.0 g/dL		01/18/2024 7:25 AM PST	MERCY LAB CV	
RDW	14.0	11.7 - 14.9 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Platelet Count	450	150 - 425 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
MPV	9.5	9.0 - 12.8 fL		01/18/2024 7:25 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/18/2024 7:25 AM PST	MERCY LAB CV	
Neutrophils %	80.2	38.0 - 74.0 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
IG%	1.0	0.0 - 1.2 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Lymphocytes %	11.8	16.0 - 48.0 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Monocytes %	6.3	4.9 - 12.5 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Eosinophils %	0.3	0.0 - 9.5 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Basophils %	0.4	0.0 - 1.6 %		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Neutrophils	9.27	1.50 - 7.40 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
IG#	0.12	0.00 - 0.10 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Lymphocytes	1.36	0.90 - 3.10 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Monocytes	0.73	0.26 - 0.87 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.04	0.00 - 0.51 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	
Preliminary Abs Neut	9.27	1.50 - 7.40 K/mcL		01/18/2024 7:25 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/18/2024 5:19 AM PST	01/18/2024 5:38 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/18/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/18/2024 6:06 AM PST	MERCY LAB CV	
Glucose	91	70 - 125 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
CO2, Total	30	22 - 30 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Urea Nitrogen	14	7 - 17 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Creatinine	0.60	0.50 - 1.00 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	103	>60 mL/min/1.73m*2		01/18/2024 6:06 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Anion Gap	1	6 - 14 mmol/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Osmolality Calculated	274	280 - 305 mOsm/kg H20		01/18/2024 6:06 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Protein, total	6.5	6.3 - 8.2 g/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	
Alkaline Phosphatase	70	38 - 126 Units/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
AST	24	14 - 36 Units/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
ALT	16	<=34 Units/L		01/18/2024 6:06 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/18/2024 6:06 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/18/2024 5:19 AM PST	01/18/2024 5:38 AM PST

Narrative

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MERCY LAB CV - 01/18/2024 6:06 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (01/17/2024 5:22 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	97	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	97	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	160	ms			SCRIPPS RADIOLOGY	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
QRS-INTERVAL (MSEC)	96	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	338	ms			SCRIPPS RADIOLOGY	
QTC	429	ms			SCRIPPS RADIOLOGY	
P AXIS	63	degrees			SCRIPPS RADIOLOGY	
R AXIS	44	degrees			SCRIPPS RADIOLOGY	
T AXIS	101	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/17/2024 5:22 PM PST	01/25/2024 8:00 AM PST

Impressions

SCRIPPS RADIOLOGY - 01/25/2024 8:00 AM PST
Normal sinus rhythm
Nonspecific T wave abnormality
Abnormal ECG

Narrative

This result has an attachment that is not available.

Procedure Note

Raghava Gollapudi, MD - 01/25/2024
Formatting of this note might be different from the original.
IMPRESSION:
Normal sinus rhythm
Nonspecific T wave abnormality
Abnormal ECG

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/17/2024 6:43 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/17/2024 9:10 AM PST	

Impressions

01/17/2024 9:11 AM PST
IMPRESSION: Stable exam, overall no significant change

release

Narrative

01/17/2024 9:11 AM PST

Narrative

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/17/2024 6:43 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/16/2024 chest x-ray and chest CT

FINDINGS:

Right chest tube in place with ongoing small right effusion. No pneumothorax. Stable cardiac contour and pulmonary vessels. Hazy opacity right base.

Procedure Note

Trevor Nelson, MD - 01/17/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/17/2024 6:43 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/16/2024 chest x-ray and chest CT

FINDINGS:

Right chest tube in place with ongoing small right effusion. No pneumothorax. Stable cardiac contour and pulmonary vessels. Hazy opacity right base.

IMPRESSION:
IMPRESSION: Stable exam, overall no significant change

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

CBC WITH DIFFERENTIAL - Final result (01/17/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	12.8	3.4 - 11.0 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
RBC	3.36	3.98 - 5.25 M/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
HGB	10.4	11.9 - 15.3 g/dL		01/17/2024 6:07 AM PST	MERCY LAB CV	
HCT	34.4	37.3 - 46.7 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/17/2024 6:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCH	31	26 - 33 pg		01/17/2024 6:07 AM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		01/17/2024 6:07 AM PST	MERCY LAB CV	
RDW	14.0	11.7 - 14.9 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Platelet Count	509	150 - 425 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
MPV	9.1	9.0 - 12.8 fL		01/17/2024 6:07 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/17/2024 6:07 AM PST	MERCY LAB CV	
Neutrophils %	86.1	38.0 - 74.0 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
IG%	0.9	0.0 - 1.2 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Lymphocytes %	7.6	16.0 - 48.0 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Monocytes %	4.9	4.9 - 12.5 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Eosinophils %	0.2	0.0 - 9.5 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Basophils %	0.3	0.0 - 1.6 %		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	11.00	1.50 - 7.40 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
IG#	0.12	0.00 - 0.10 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Lymphocytes	0.97	0.90 - 3.10 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.62	0.26 - 0.87 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.03	0.00 - 0.51 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	11.00	1.50 - 7.40 K/mcL		01/17/2024 6:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/17/2024 5:19 AM PST	01/17/2024 5:58 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/17/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/17/2024 6:19 AM PST	MERCY LAB CV	
Glucose	99	70 - 125 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Potassium	4.3	3.5 - 5.1 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Urea Nitrogen	19	7 - 17 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/17/2024 6:19 AM PST	MERCY LAB CV	
Anion Gap	1	6 - 14 mmol/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Osmolality Calculated	276	280 - 305 mOsm/kg H20		01/17/2024 6:19 AM PST	MERCY LAB CV	
Calcium	8.6	8.4 - 10.2 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Protein, total	6.1	6.3 - 8.2 g/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Albumin	2.5	3.5 - 5.0 g/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	
Alkaline Phosphatase	77	38 - 126 Units/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
AST	24	14 - 36 Units/L		01/17/2024 6:19 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
ALT	16	<=34 Units/L		01/17/2024 6:19 AM PST	MERCY LAB CV	
Bilirubin, Total	0.3	0.2 - 1.3 mg/dL		01/17/2024 6:19 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/17/2024 5:19 AM PST	01/17/2024 5:56 AM PST

Narrative

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MERCY LAB CV - 01/17/2024 6:19 AM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINE CULTURE - Final result (01/16/2024 2:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Urine Culture	Multiple organisms present, probable skin/ urogenital contamination.		MIC	01/18/2024 11:59 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/16/2024 2:33 PM PST	01/16/2024 3:42 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

URINALYSIS MICROSCOPIC W/ REFLEX TO CULTURE - Final result (01/16/2024 2:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC (UA)	0-2	0 - 2 / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
RBC (UA)	0-2	0 - 2 / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
Epithelial Cells, Squamous (UA)	Few	None / LPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
Bacteria (UA)	Few	None / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
Amorphous Urates Crystals (UA)	Few	None / LPF		01/16/2024 3:56 PM PST	MERCY LAB CV	
Budding Yeast (UA)	Few	None / HPF		01/16/2024 3:56 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/16/2024 2:33 PM PST	01/16/2024 3:28 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS, MACROSCOPIC, W/REFLEX TO CULTURE (URINALYSIS W/REFLEX TO CULTURE) - Final result (01/16/2024 2:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Color (UA)	Yellow	Yellow		01/16/2024 3:43 PM PST	MERCY LAB CV	
Clarity (UA)	Clear	Clear		01/16/2024 3:43 PM PST	MERCY LAB CV	
Specific Gravity (UA)	1.025	1.005 - 1.030		01/16/2024 3:43 PM PST	MERCY LAB CV	
pH (UA)	6.0	5.0 - 8.5 pH		01/16/2024 3:43 PM PST	MERCY LAB CV	
Protein (UA)	30	Negative mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	
Glucose (UA)	Negative	Negative mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	
Ketones (UA)	Negative	Negative mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	
Bilirubin (UA)	Negative	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Occult Blood (UA)	Negative	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	
Leukocyte Esterase (UA)	Negative	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	
Nitrite (UA)	Positive	Negative		01/16/2024 3:43 PM PST	MERCY LAB CV	
Urobilinogen (UA)	0.2	<2.0 mg/dL		01/16/2024 3:43 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/16/2024 2:33 PM PST	01/16/2024 3:28 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (01/16/2024 11:25 AM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/16/2024 11:52 AM PST	

Impressions

01/16/2024 11:52 AM PST
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Narrative

01/16/2024 11:52 AM PST
EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/16/2024 11:25 AM

HISTORY: RULE OUT DVT

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

Procedure Note

Harold Pimenta, MD - 01/16/2024

Formatting of this note might be different from the original.

EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/16/2024 11:25 AM

HISTORY: RULE OUT DVT

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: None.

IMPRESSION:
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

release

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	IMG US PROCEDURES	Final Result

CT chest without contrast (CT CHEST WITHOUT CONTRAST) - Final result (01/16/2024 9:51 AM PST)

Anatomical Region	Laterality	Modality
Chest		Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/16/2024 9:57 AM PST	

Impressions

01/16/2024 10:07 AM PST

IMPRESSION:

1. Small to moderate right pleural effusion with indwelling pleural drainage catheter. Subjacent right basilar atelectasis/consolidation

2. Right upper lobe groundglass opacity with peripheral consolidation (reverse halo sign). Findings can be seen in the setting of atypical community-acquired bacterial pneumonia, fungal pneumonia, and organizing pneumonia.

release

Narrative

01/16/2024 10:07 AM PST

Narrative

EXAMINATION: CT CHEST WITHOUT CONTRAST

DATE/TIME: 1/16/2024 9:51 AM

HISTORY: 59 years old Female, Pneumonia

COMPARISON: Same day chest radiograph.

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

Lungs: Small to moderate right pleural effusion with indwelling pleural drainage catheter and scattered expected foci of pleural air. Subjacent right basilar atelectasis/consolidation. Right upper lobe groundglass opacity with mild peripheral consolidation (series 5/102). Multifocal right middle and lower lobe scarring and volume loss.

Airways: Patent trachea and central airways. Mild diffuse bronchial wall thickening.

Vasculature: Thoracic aortic atherosclerosis without aneurysm. Main pulmonary artery enlargement up to 3.8 cm.

Cardiac: Mild coronary artery calcifications. Mild cardiomegaly. No pericardial effusion.

Lymph nodes: Limited evaluation without IV contrast. No bulky supraclavicular, axillary, mediastinal, or hilar lymphadenopathy.

Other mediastinal structures: Mild esophageal wall thickening, which may be due to underdistention or esophagitis.

Chest wall: No significant abnormality.

Bones: Old healed midsternal fracture. Degenerative changes of the spine. No aggressive osseous lesion.

Upper abdomen: No significant abnormality.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: Chest; Phantom: 32 cm; CTDIvol: 5; DLP: 177

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

Procedure Note

Pranay Kothari, MD - 01/16/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: CT CHEST WITHOUT CONTRAST

DATE/TIME: 1/16/2024 9:51 AM

HISTORY: 59 years old Female, Pneumonia

COMPARISON: Same day chest radiograph.

TECHNIQUE: CT examination was performed without intravenous contrast.
In accordance with CT protocols and the ALARA principle, radiation dose reduction techniques were utilized for this examination.

FINDINGS:

Lungs: Small to moderate right pleural effusion with indwelling pleural drainage catheter and scattered expected foci of pleural air. Subjacent right basilar atelectasis/consolidation. Right upper lobe groundglass opacity with mild peripheral consolidation (series 5/102). Multifocal right middle and lower lobe scarring and volume loss.

Airways: Patent trachea and central airways. Mild diffuse bronchial wall thickening.

Vasculature: Thoracic aortic atherosclerosis without aneurysm. Main pulmonary artery enlargement up to 3.8 cm.

Cardiac: Mild coronary artery calcifications. Mild cardiomegaly. No pericardial effusion.

Lymph nodes: Limited evaluation without IV contrast. No bulky supraclavicular, axillary, mediastinal, or hilar lymphadenopathy.

Other mediastinal structures: Mild esophageal wall thickening, which may be due to underdistention or esophagitis.

Chest wall: No significant abnormality.

Bones: Old healed midsternal fracture. Degenerative changes of the spine. No aggressive osseous lesion.

Upper abdomen: No significant abnormality.

EXPOSURE:
This patient received a total of 1 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 3; Anatomy: Chest; Phantom: 32 cm; CTDIvol: 5; DLP: 177

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient.

IMPRESSION:
IMPRESSION:

1. Small to moderate right pleural effusion with indwelling pleural drainage catheter. Subjacent right basilar atelectasis/ consolidation

2. Right upper lobe groundglass opacity with peripheral consolidation (reverse halo sign). Findings can be seen in the setting of atypical community-acquired bacterial pneumonia, fungal pneumonia, and organizing pneumonia.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG CT PROCEDURES	Final Result

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/16/2024 6:27 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/16/2024 9:35 AM PST	

Impressions

01/16/2024 9:35 AM PST
IMPRESSION: No significant interval change.

release
CC Dr: p: f:

Narrative

01/16/2024 9:35 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/16/2024 6:27 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/15/2024

FINDINGS: Stable position of the right chest tube. Moderate volume right pleural effusion/thickening is seen with mild to moderate patchy and streak-like change of the right lung base. Stable cardiomegaly with mild bronchovascular prominence. No left lung infiltrate or clear-cut pneumothorax.

Procedure Note

Ross Christensen, MD - 01/16/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/16/2024 6:27 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/15/2024

FINDINGS: Stable position of the right chest tube. Moderate volume right pleural effusion/thickening is seen with mild to moderate patchy and streak-like change of the right lung base. Stable cardiomegaly with mild bronchovascular prominence. No left lung infiltrate or clear-cut pneumothorax.

IMPRESSION:
IMPRESSION: No significant interval change.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

COMPREHENSIVE METABOLIC PANEL - Final result (01/16/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/16/2024 9:33 AM PST	MERCY LAB CV	
Glucose	116	70 - 125 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	131	137 - 145 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
CO2, Total	37	22 - 30 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Urea Nitrogen	20	7 - 17 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Creatinine	0.70	0.50 - 1.00 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	100	>60 mL/min/1.73m*2		01/16/2024 9:33 AM PST	MERCY LAB CV	
Anion Gap	-6	6 - 14 mmol/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Osmolality Calculated	276	280 - 305 mOsm/kg H20		01/16/2024 9:33 AM PST	MERCY LAB CV	
Calcium	8.5	8.4 - 10.2 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Protein, total	6.9	6.3 - 8.2 g/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Albumin	2.8	3.5 - 5.0 g/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	
Alkaline Phosphatase	95	38 - 126 Units/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
AST	102	14 - 36 Units/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
ALT	19	<=34 Units/L		01/16/2024 9:33 AM PST	MERCY LAB CV	
Bilirubin, Total	0.4	0.2 - 1.3 mg/dL		01/16/2024 9:33 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/16/2024 5:19 AM PST	01/16/2024 6:01 AM PST

Narrative

MERCY LAB CV - 01/16/2024 9:33 AM PST

Narrative

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for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Amanda de Four MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/16/2024 5:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	13.1	3.4 - 11.0 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
RBC	3.37	3.98 - 5.25 M/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
HGB	10.6	11.9 - 15.3 g/dL		01/16/2024 5:49 AM PST	MERCY LAB CV	
HCT	35.1	37.3 - 46.7 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
MCV	104	81 - 100 fL		01/16/2024 5:49 AM PST	MERCY LAB CV	
MCH	32	26 - 33 pg		01/16/2024 5:49 AM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		01/16/2024 5:49 AM PST	MERCY LAB CV	
RDW	14.5	11.7 - 14.9 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Platelet Count	546	150 - 425 K/mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
MPV	9.3	9.0 - 12.8 fL		01/16/2024 5:49 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/16/2024 5:49 AM PST	MERCY LAB CV	
Neutrophils %	85.4	38.0 - 74.0 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
IG%	2.0	0.0 - 1.2 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Lymphocytes %	8.1	16.0 - 48.0 %		01/16/2024 5:49 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	4.1	4.9 - 12.5 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Eosinophils %	0.2	0.0 - 9.5 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Neutrophils	11.21	1.50 - 7.40 K/ mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
IG#	0.26	0.00 - 0.10 K/ mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.07	0.90 - 3.10 K/ mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Monocytes	0.54	0.26 - 0.87 K/ mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.02	0.00 - 0.51 K/ mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/ mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	
Preliminary Abs Neut	11.21	1.50 - 7.40 K/ mcL		01/16/2024 5:49 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/16/2024 5:19 AM PST	01/16/2024 5:40 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray chest single frontal view (X-RAY CHEST SINGLE FRONTAL VIEW) - Final result (01/15/2024 11:42 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/15/2024 11:43 AM PST	

Impressions

01/15/2024 11:44 AM PST

Impressions

IMPRESSION:
1. Worsening right basilar atelectasis/consolidation.
2. Persistent right pleural effusion.
3. Right chest tube is unchanged. No pneumothorax.

release
CC Dr: p: f:

Narrative

01/15/2024 11:44 AM PST
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/15/2024 11:42 AM

HISTORY: Respiratory failure

COMPARISON: 1/14/2024

FINDINGS:
Endotracheal tube has been removed.

Right chest tube is unchanged.

There is no significant pneumothorax.

Increase in right basilar hazy and linear pulmonary opacification. Right costophrenic angle remains blunted.

Left costophrenic angle is sharp.

There is no evidence of pneumothorax.

Cardiomegaly.

Procedure Note

Stephen Hulse, MD - 01/15/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/15/2024 11:42 AM

HISTORY: Respiratory failure

COMPARISON: 1/14/2024

FINDINGS:
Endotracheal tube has been removed.

Right chest tube is unchanged.

There is no significant pneumothorax.

Increase in right basilar hazy and linear pulmonary opacification. Right costophrenic angle remains blunted.

Left costophrenic angle is sharp.

There is no evidence of pneumothorax.

Cardiomegaly.

IMPRESSION:
IMPRESSION:
1. Worsening right basilar atelectasis/consolidation.
2. Persistent right pleural effusion.
3. Right chest tube is unchanged. No pneumothorax.

release
CC Dr: p: f:

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

GLUCOSE, POC - Final result (01/15/2024 5:52 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	97	70 - 125 mg/dL		01/15/2024 5:54 AM PST	MERCY LAB CV	
Operator ID	233,925			01/15/2024 5:54 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/15/2024 5:52 AM PST	01/15/2024 5:54 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Martha Strauser MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/15/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	12.5	3.4 - 11.0 K/mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
RBC	3.31	3.98 - 5.25 M/mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
HGB	10.4	11.9 - 15.3 g/dL		01/15/2024 5:47 AM PST	MERCY LAB CV	
HCT	33.9	37.3 - 46.7 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
MCV	102	81 - 100 fL		01/15/2024 5:47 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/15/2024 5:47 AM PST	MERCY LAB CV	
MCHC	30.7	31.0 - 36.0 g/dL		01/15/2024 5:47 AM PST	MERCY LAB CV	
RDW	14.6	11.7 - 14.9 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Platelet Count	543	150 - 425 K/mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
MPV	9.3	9.0 - 12.8 fL		01/15/2024 5:47 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
nRBC	0.0	0.0 /100 WBCs		01/15/2024 5:47 AM PST	MERCY LAB CV	
Neutrophils %	85.7	38.0 - 74.0 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
IG%	2.6	0.0 - 1.2 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Lymphocytes %	7.5	16.0 - 48.0 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Monocytes %	3.8	4.9 - 12.5 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Eosinophils %	0.2	0.0 - 9.5 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Neutrophils	10.68	1.50 - 7.40 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
IG#	0.33	0.00 - 0.10 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Lymphocytes	0.93	0.90 - 3.10 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Monocytes	0.48	0.26 - 0.87 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.02	0.00 - 0.51 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	
Preliminary Abs Neut	10.68	1.50 - 7.40 K/ mcL		01/15/2024 5:47 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/15/2024 5:28 AM PST	01/15/2024 5:43 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (01/15/2024 5:28 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/15/2024 6:12 AM PST	MERCY LAB CV	
Glucose	102	70 - 125 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	
Sodium	135	137 - 145 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
CO2, Total	34	22 - 30 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Urea Nitrogen	33	7 - 17 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	
Creatinine	0.90	0.50 - 1.00 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	74	>60 mL/min/1.73m*2		01/15/2024 6:12 AM PST	MERCY LAB CV	
Anion Gap	-1	6 - 14 mmol/L		01/15/2024 6:12 AM PST	MERCY LAB CV	
Osmolality Calculated	287	280 - 305 mOsm/kg H20		01/15/2024 6:12 AM PST	MERCY LAB CV	
Calcium	8.4	8.4 - 10.2 mg/dL		01/15/2024 6:12 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/15/2024 5:28 AM PST	01/15/2024 5:44 AM PST

Narrative

MERCY LAB CV - 01/15/2024 6:12 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL: Normal
100-125 mg/dL: Impaired Fasting Glucose
>125 mg/dL: Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/15/2024 12:15 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	103	70 - 125 mg/dL		01/15/2024 12:19 AM PST	MERCY LAB CV	
Operator ID	233,925			01/15/2024 12:19 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/15/2024 12:15 AM PST	01/15/2024 12:19 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Truc Ho MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 5:17 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	93	70 - 125 mg/dL		01/14/2024 5:19 PM PST	MERCY LAB CV	
Operator ID	244,516			01/14/2024 5:19 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 5:17 PM PST	01/14/2024 5:19 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 12:29 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	101	70 - 125 mg/dL		01/14/2024 12:32 PM PST	MERCY LAB CV	
Operator ID	145,602			01/14/2024 12:32 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 12:29 PM PST	01/14/2024 12:32 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 6:11 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	94	70 - 125 mg/dL		01/14/2024 6:19 AM PST	MERCY LAB CV	
Operator ID	232,690			01/14/2024 6:19 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 6:11 AM PST	01/14/2024 6:19 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/14/2024 7:41 AM PST	MERCY LAB CV	
Glucose	144	70 - 125 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	
Sodium	133	137 - 145 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Chloride	104	98 - 107 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
CO2, Total	26	22 - 30 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Urea Nitrogen	50	7 - 17 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	
Creatinine	1.20	0.50 - 1.00 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	52	>60 mL/min/1.73m*2		01/14/2024 7:41 AM PST	MERCY LAB CV	
Anion Gap	3	6 - 14 mmol/L		01/14/2024 7:41 AM PST	MERCY LAB CV	
Osmolality Calculated	292	280 - 305 mOsm/kg H20		01/14/2024 7:41 AM PST	MERCY LAB CV	
Calcium	8.1	8.4 - 10.2 mg/dL		01/14/2024 7:41 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

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MERCY LAB CV - 01/14/2024 7:41 AM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
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Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	14.3	3.4 - 11.0 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
RBC	3.14	3.98 - 5.25 M/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
HGB	9.7	11.9 - 15.3 g/dL		01/14/2024 5:52 AM PST	MERCY LAB CV	
HCT	30.4	37.3 - 46.7 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
MCV	97	81 - 100 fL		01/14/2024 5:52 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/14/2024 5:52 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCHC	31.9	31.0 - 36.0 g/dL		01/14/2024 5:52 AM PST	MERCY LAB CV	
RDW	14.6	11.7 - 14.9 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Platelet Count	573	150 - 425 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
MPV	9.6	9.0 - 12.8 fL		01/14/2024 5:52 AM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/14/2024 5:52 AM PST	MERCY LAB CV	
Neutrophils %	84.2	38.0 - 74.0 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
IG%	2.8	0.0 - 1.2 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Lymphocytes %	9.7	16.0 - 48.0 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Monocytes %	3.0	4.9 - 12.5 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Eosinophils %	0.1	0.0 - 9.5 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Basophils %	0.2	0.0 - 1.6 %		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Neutrophils	12.01	1.50 - 7.40 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
IG#	0.40	0.00 - 0.10 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.39	0.90 - 3.10 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Monocytes	0.43	0.26 - 0.87 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.01	0.00 - 0.51 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Absolute Basophils	0.03	0.00 - 0.09 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	
Preliminary Abs Neut	12.01	1.50 - 7.40 K/mcL		01/14/2024 5:52 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:22 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

VANCOMYCIN LEVEL, RANDOM - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Vancomycin Level, Random	16	Not Established mcg/mL		01/14/2024 5:50 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MAGNESIUM - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Magnesium	2.3	1.6 - 2.3 mg/dL		01/14/2024 5:45 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

MERCY LAB CV - 01/14/2024 5:45 AM PST

NOTE: Higher values can be expected in females during menses.

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CK - Final result (01/14/2024 5:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
CK	128	30 - 135 Units/L		01/14/2024 5:45 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 5:02 AM PST	01/14/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/14/2024 4:29 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/14/2024 9:20 AM PST	

Impressions

01/14/2024 9:22 AM PST
IMPRESSION:
Essentially unchanged examination.

release

Narrative

01/14/2024 9:22 AM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/14/2024 4:29 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/13/2024

FINDINGS:
One view obtained.

Right basilar atelectasis/consolidation. There is a small right pleural effusion. Right chest tube remains in place. No appreciable pneumothorax. Left lung appears clear.

The cardiomediastinal silhouette is enlarged. Tip of the endotracheal tube projects 4.1 cm superior to the carina. Enteric tube passes below the diaphragm, tip not seen. The osseous structures demonstrate no acute abnormality.

Procedure Note

Anshuman Bansal, MD - 01/14/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/14/2024 4:29 AM

HISTORY: Respiratory failure, not elsewhere classified

COMPARISON: 1/13/2024

FINDINGS:
One view obtained.

Right basilar atelectasis/consolidation. There is a small right pleural effusion. Right chest tube remains in place. No appreciable pneumothorax. Left lung appears clear.

The cardiomediastinal silhouette is enlarged. Tip of the endotracheal tube projects 4.1 cm superior to the carina. Enteric tube passes below the diaphragm, tip not seen. The osseous structures demonstrate no acute abnormality.

IMPRESSION:
IMPRESSION:
Essentially unchanged examination.

release

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

BLOOD GAS LACTIC ACID, ARTERIAL - Final result (01/14/2024 3:47 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.45	7.35 - 7.45		01/14/2024 4:06 AM PST	MERCY LAB CV	
pCO2, Arterial	42	35 - 45 mm Hg		01/14/2024 4:06 AM PST	MERCY LAB CV	
pO2, Arterial	56	83 - 108 mm Hg		01/14/2024 4:06 AM PST	MERCY LAB CV	
HCO3, Arterial	29.4	21.0 - 28.0 mmol/L		01/14/2024 4:06 AM PST	MERCY LAB CV	
O2 Sat, Arterial	90.1	95.0 - 98.0 %		01/14/2024 4:06 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	5.5	-2.0 - 3.0 mmol/L		01/14/2024 4:06 AM PST	MERCY LAB CV	
FIO2, Arterial	35.0			01/14/2024 4:06 AM PST	MERCY LAB CV	
Oxygen Device	Ventilator			01/14/2024 4:06 AM PST	MERCY LAB CV	
Modes	PRVC			01/14/2024 4:06 AM PST	MERCY LAB CV	
Lactic Acid, Arterial	1.0	0.4 - 0.9 mmol/L		01/14/2024 4:06 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Allen test	NA			01/14/2024 4:06 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/14/2024 4:06 AM PST	MERCY LAB CV	
Comment	400/20/5			01/14/2024 4:06 AM PST	MERCY LAB CV	
Notified By	243560			01/14/2024 4:06 AM PST	MERCY LAB CV	
Notified Whom	258646			01/14/2024 4:06 AM PST	MERCY LAB CV	
Operator ID	243,560			01/14/2024 4:06 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/14/2024 3:47 AM PST	01/14/2024 4:06 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/14/2024 12:22 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	173	70 - 125 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Sodium	134	137 - 145 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Potassium	4.6	3.5 - 5.1 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Chloride	104	98 - 107 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
CO2, Total	28	22 - 30 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Urea Nitrogen	53	7 - 17 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Creatinine	1.22	0.50 - 1.00 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	51	>60 mL/min/1.73m*2		01/14/2024 12:41 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Anion Gap	2	6 - 14 mmol/L		01/14/2024 12:41 AM PST	MERCY LAB CV	
Osmolality Calculated	297	280 - 305 mOsm/kg H20		01/14/2024 12:41 AM PST	MERCY LAB CV	
Calcium	8.3	8.4 - 10.2 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Phosphorus	2.9	2.5 - 4.5 mg/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	
Albumin	2.6	3.5 - 5.0 g/dL		01/14/2024 12:41 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/14/2024 12:22 AM PST	01/14/2024 12:26 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/14/2024 12:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	94	70 - 125 mg/dL		01/14/2024 12:21 AM PST	MERCY LAB CV	
Operator ID	234,267			01/14/2024 12:21 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/14/2024 12:19 AM PST	01/14/2024 12:21 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LEGIONELLA ANTIGEN URINE (LEGIONELLA ANTIGEN, URINE) - Final result (01/13/2024 11:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Legionella Ag, Urine	Negative	Negative		01/14/2024 3:48 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown	Non-Lab Collection / Unknown	01/13/2024 11:41 PM PST	01/13/2024 11:58 PM PST

Narrative

SCRIPPS LAB MICROBIOLOGY - 01/14/2024 3:48 PM PST
Negative (presumptive) for Legionella pneumophilia, serogroup 1 antigen. Suggests no recent or current infection. Infection cannot be ruled out as antigen may not be present in urine in early infection or the level of antigen may be below detectable limits.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

STREP PNEUMO ANTIGEN, URINE - Final result (01/13/2024 11:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Strep Pneumo Ag,UR	Positive	Negative		01/14/2024 3:48 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown	Non-Lab Collection / Unknown	01/13/2024 11:41 PM PST	01/13/2024 11:58 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

CULTURE, RESPIRATORY (RESPIRATORY CULTURE WITH STAIN) - Final result (01/13/2024 9:40 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Culture	No Growth Final		MIC	01/16/2024 8:09 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	Moderate WBCs			01/16/2024 8:09 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	No organisms seen			01/16/2024 8:09 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Sputum	Sputum / Unknown	Non-Lab Collection / Unknown	01/13/2024 9:40 PM PST	01/13/2024 9:43 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/13/2024 8:28 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	135	70 - 125 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Sodium	132	137 - 145 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Potassium	4.7	3.5 - 5.1 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Chloride	103	98 - 107 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
CO2, Total	27	22 - 30 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Urea Nitrogen	56	7 - 17 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Creatinine	1.30	0.50 - 1.00 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	47	>60 mL/min/1.73m*2		01/13/2024 8:50 PM PST	MERCY LAB CV	
Anion Gap	2	6 - 14 mmol/L		01/13/2024 8:50 PM PST	MERCY LAB CV	
Osmolality Calculated	292	280 - 305 mOsm/kg H20		01/13/2024 8:50 PM PST	MERCY LAB CV	
Calcium	8.0	8.4 - 10.2 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Phosphorus	3.5	2.5 - 4.5 mg/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		01/13/2024 8:50 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 8:28 PM PST	01/13/2024 8:31 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE, POC - Final result (01/13/2024 6:07 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	99	70 - 125 mg/dL		01/13/2024 6:09 PM PST	MERCY LAB CV	
Operator ID	155,359			01/13/2024 6:09 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 6:07 PM PST	01/13/2024 6:09 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (01/13/2024 3:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/13/2024 3:51 PM PST	MERCY LAB CV	
Glucose	132	70 - 125 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Potassium	5.3	3.5 - 5.1 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
CO2, Total	25	22 - 30 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Urea Nitrogen	59	7 - 17 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Creatinine	1.20	0.50 - 1.00 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	52	>60 mL/min/1.73m*2		01/13/2024 3:51 PM PST	MERCY LAB CV	
Anion Gap	6	6 - 14 mmol/L		01/13/2024 3:51 PM PST	MERCY LAB CV	
Osmolality Calculated	290	280 - 305 mOsm/kg H20		01/13/2024 3:51 PM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		01/13/2024 3:51 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 3:33 PM PST	01/13/2024 3:36 PM PST

Narrative

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MERCY LAB CV - 01/13/2024 3:51 PM PST
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BETA HYDROXYBUTYRATE - Final result (01/13/2024 3:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Beta-Hydroxybutyrate	2.2	0.2 - 2.8 mg/dL		01/13/2024 3:59 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 3:33 PM PST	01/13/2024 3:36 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

VANCOMYCIN LEVEL, RANDOM - Final result (01/13/2024 3:33 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Vancomycin Level, Random	14	Not Established mcg/mL		01/13/2024 3:56 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 3:33 PM PST	01/13/2024 3:36 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

Ultrasound abdominal limited liver (ULTRASOUND ABDOMEN LIMITED LIVER) - Final result (01/13/2024 2:52 PM PST)

Anatomical Region	Laterality	Modality
Body		Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 2:54 PM PST	

Impressions

01/13/2024 2:55 PM PST
IMPRESSION: Unremarkable gallbladder.

Echogenic liver. Consider steatosis or hepatocellular disease.

Right pleural effusion

release

Narrative

01/13/2024 2:55 PM PST
EXAMINATION: ULTRASOUND ABDOMEN LIMITED LIVER

DATE/TIME: 1/13/2024 2:52 PM

HISTORY: Reported abdominal pain, please also evaluate gallbladder

COMPARISON: None.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: Length is 19.1 cm. Increased echogenicity. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
BILIARY AND GALLBLADDER: Normal gallbladder. No ductal dilatation. CBD diameter is 5.1 mm.
PANCREAS: Not well visualized
RIGHT KIDNEY: Right renal length is 10.0 cm. No visible solid mass, calculus or hydronephrosis.
AORTA/IVC/VASC: No visible aneurysm. The intrahepatic IVC is patent
OTHER: Right pleural effusion

Procedure Note

Raymond Hong, MD - 01/13/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: ULTRASOUND ABDOMEN LIMITED LIVER

DATE/TIME: 1/13/2024 2:52 PM

HISTORY: Reported abdominal pain, please also evaluate gallbladder

COMPARISON: None.

TECHNIQUE: High resolution sonographic examination was performed of the abdomen in the limited region as described below.

FINDINGS:
LIVER: Length is 19.1 cm. Increased echogenicity. No visible mass.
PORTAL VEIN: The portal vein is patent with normal antegrade flow.
BILIARY AND GALLBLADDER: Normal gallbladder. No ductal dilatation. CBD diameter is 5.1 mm.
PANCREAS: Not well visualized
RIGHT KIDNEY: Right renal length is 10.0 cm. No visible solid mass, calculus or hydronephrosis.
AORTA/IVC/VASC: No visible aneurysm. The intrahepatic IVC is patent
OTHER: Right pleural effusion

IMPRESSION:
IMPRESSION: Unremarkable gallbladder.

Echogenic liver. Consider steatosis or hepatocellular disease.

Right pleural effusion

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG US PROCEDURES	Final Result

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/13/2024 12:38 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	347	70 - 125 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Sodium	131	137 - 145 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Potassium	5.8	3.5 - 5.1 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Chloride	102	98 - 107 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
CO2, Total	20	22 - 30 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Urea Nitrogen	55	7 - 17 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Creatinine	1.10	0.50 - 1.00 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	58	>60 mL/min/1.73m*2		01/13/2024 1:14 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Anion Gap	9	6 - 14 mmol/L		01/13/2024 1:14 PM PST	MERCY LAB CV	
Osmolality Calculated	301	280 - 305 mOsm/kg H20		01/13/2024 1:14 PM PST	MERCY LAB CV	
Calcium	7.9	8.4 - 10.2 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Phosphorus	5.7	2.5 - 4.5 mg/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	
Albumin	2.7	3.5 - 5.0 g/dL		01/13/2024 1:14 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 12:38 PM PST	01/13/2024 12:48 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/13/2024 12:09 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 12:16 PM PST	

Impressions

01/13/2024 12:17 PM PST
IMPRESSION: Interval decrease in moderate right pleural effusion status post chest tube insertion. No pneumothorax.

release

Narrative

01/13/2024 12:17 PM PST

Narrative

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 12:09 PM

HISTORY: s/p chest tube insertion

COMPARISON: 1/13/2024

FINDINGS:

New right chest tube. Endotracheal and gastric tubes are in stable position.

The heart and mediastinum are unchanged.

Interval decrease in moderate right pleural effusion. No pneumothorax. Right basilar atelectasis or infiltrate again noted.

Imaged skeletal structures are unchanged.

Procedure Note

Raymond Hong, MD - 01/13/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 12:09 PM

HISTORY: s/p chest tube insertion

COMPARISON: 1/13/2024

FINDINGS:

New right chest tube. Endotracheal and gastric tubes are in stable position.

The heart and mediastinum are unchanged.

Interval decrease in moderate right pleural effusion. No pneumothorax. Right basilar atelectasis or infiltrate again noted.

Imaged skeletal structures are unchanged.

IMPRESSION:
IMPRESSION: Interval decrease in moderate right pleural effusion status post chest tube insertion. No pneumothorax.

release

Authorizing Provider	Result Type	Result Status
Brandon May DNP	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

GLUCOSE, POC - Final result (01/13/2024 12:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	111	70 - 125 mg/dL		01/13/2024 4:12 PM PST	MERCY LAB CV	
Operator ID	244,516			01/13/2024 4:12 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 12:03 PM PST	01/13/2024 4:12 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CHEST TUBE INSERTION (01/13/2024 11:44 AM PST)

Narrative

Pedro Dammert Coronado, MD - 01/13/2024 11:44 AM PST
May, Brandon Michael, DNP 1/13/2024 11:46 AM
Chest Tube Insertion

Date/Time: 1/13/2024 11:44 AM

Performed by: May, Brandon Michael, DNP
Authorized by: May, Brandon Michael, DNP
Consent:
 Consent obtained: Written
 Consent given by: Healthcare agent
 Risks, benefits, and alternatives were discussed: yes
 Risks discussed: Bleeding, damage to surrounding structures, nerve damage, pain, infection and incomplete drainage
 Alternatives discussed: No treatment
Universal protocol:
 Procedure explained and questions answered to patient or proxy's satisfaction: yes
 Relevant documents present and verified: yes
 Test results available: yes
 Imaging studies available: yes
 Required blood products, implants, devices, and special equipment available: yes
 Site/side marked: yes
 Immediately prior to procedure, a time out was called: yes
 Patient identity confirmed: Hospital-assigned identification number and arm band
Pre-procedure details:
 Skin preparation: Chlorhexidine with alcohol
 Preparation: Patient was prepped and draped in the usual sterile fashion

Anesthesia:
 Anesthesia method: Local infiltration
 Local anesthetic: Lidocaine 1% w/o epi
Procedure details:
 Placement location: R lateral
 Tube size (French): 14.
 Ultrasound guidance: yes
 Tension pneumothorax: no
 Tube connected to: Water seal
 Drainage characteristics: Purulent and yellow
 Suture material: 2-0 silk
 Dressing: Tegaderm.
Post-procedure details:
 Procedure completion: Tolerated well, no immediate complications
Comments:
 CXR pending, Dr. Dammert at bedside for the entire procedure

Authorizing Provider	Result Type	Result Status
Brandon May DNP	PROCEDURE/MINOR SURGICAL ORDERABLES	Final Result

Vascular Ult venous duplex lower extremity complete bilateral (VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL) - Final result (01/13/2024 10:57 AM PST)

Anatomical Region	Laterality	Modality
Lower Extremities	Bilateral	Ultrasound

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 11:27 AM PST	

Impressions

01/13/2024 11:27 AM PST
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

1.7 cm right popliteal cyst

release

Narrative

01/13/2024 11:27 AM PST
EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/13/2024 10:57 AM

HISTORY: Asymmetric edema R>L

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: 1.7 cm right popliteal cyst.

Procedure Note

Raymond Hong, MD - 01/13/2024
Formatting of this note might be different from the original.
EXAMINATION: VASCULAR ULT VENOUS DUPLEX LOWER EXTREMITY COMPLETE BILATERAL

DATE/TIME: 1/13/2024 10:57 AM

HISTORY: Asymmetric edema R>L

COMPARISON: None.

TECHNIQUE: Sonographic examination was performed. Duplex scan was performed characterizing the pattern and direction of blood flow with the production of real-time images integrating B-mode two-dimensional vascular structure, Doppler spectral analysis, and color flow Doppler imaging.

FINDINGS:
VEINS: No intraluminal thrombus. Normal venous compression and normal augmentation response. Tiny peripheral calf vein thrombi may not be detectable.
OTHER: 1.7 cm right popliteal cyst.

IMPRESSION:
IMPRESSION: Negative for deep vein thrombosis in both lower extremities.

1.7 cm right popliteal cyst

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG US PROCEDURES	Final Result

2D ECHO COMPLETE WITHOUT CONTRAST (WITH COLOR AND SPECTRAL DOPPLER AND MYOCARDIAL STRAIN) (01/13/2024 9:44 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Interventricular Septal Thickness End Diastole	1.1	cm			SCRIPPS RADIOLOGY	
Left Ventricular Internal Diameter End Diastole	6.2	cm			SCRIPPS RADIOLOGY	
Left Ventricular Internal Diameter End Systole	5.4	cm			SCRIPPS RADIOLOGY	
Left Ventricular Posterior Wall	1.1	cm			SCRIPPS RADIOLOGY	
Left Ventricular End Systolic Volume Indexed	94	ml			SCRIPPS RADIOLOGY	
Left Ventricular Ejection Fraction	26.2	%			SCRIPPS RADIOLOGY	
Mitral Valve E/A Ratio	0.89				SCRIPPS RADIOLOGY	
Aortic Valve Maximum Velocity	146	cm/sec			SCRIPPS RADIOLOGY	
Aortic Valve Mean Pressure Gradient	4	mmHg			SCRIPPS RADIOLOGY	
Aortic Valve Area	2	cm^2			SCRIPPS RADIOLOGY	
LVOT VTI	18.2	cm			SCRIPPS RADIOLOGY	
Mitral Regurgitation Effective Regurgitant Orifice Area	0.13	cm^2			SCRIPPS RADIOLOGY	
Mitral Regurgitant Volume	20.9	ml			SCRIPPS RADIOLOGY	
Stroke Volume (LVOT) Indexed	26.7	ml/m^2			SCRIPPS RADIOLOGY	
RA Volume (Indexed)	36.3	ml/m^2			SCRIPPS RADIOLOGY	
Mitral Valve Average E/e' Ratio	13.2				SCRIPPS RADIOLOGY	
Left Ventricular End Diastolic Volume Indexed	73.3	ml/m^2			SCRIPPS RADIOLOGY	
Left Atrial Volume Indexed	38.3				SCRIPPS RADIOLOGY	
Ascending Aorta Indexed	2.1	cm			SCRIPPS RADIOLOGY	
Ao Sinus Indexed	2.2	cm			SCRIPPS RADIOLOGY	
Right Ventricular Diameter 2 (mid-level)	1.6	cm			SCRIPPS RADIOLOGY	
TAPSE	1.6	cm			SCRIPPS RADIOLOGY	
Dimensionless Index (LVOT/AoV)	0.81				SCRIPPS RADIOLOGY	

Anatomical Region	Laterality	Modality
		Echocardiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 8:39 AM PST	

Impressions

01/14/2024 7:37 AM PST

Study ID: 1074047

Scripps Mercy Hospital Chula Vista
Cardiology Lab
435 H Street
Chula Vista, CA 91910
(619) 691-7499

Adult Echo Report

Name: PETRUCCIO, ELIZABETH TINA
MRN: 810760324
DOB: 07/29/1964
Age: 59 yrs
Gender: Female
HR: 66
Study Date: 01/13/2024 08:39 AM
Patient Location: SVICU^0ICU^11^CV
Height: 67 in
Weight: 140 lb
BSA: 1.7 m2
BP: 105/83 mmHg
Reason For Study: Other Cardiac Reason, Specify in Comments
History: Heart failure reduced ejection fraction on echo, reported history of asthma; medical noncompliance, past alcohol and methamphetamine abuse
Acct Number: 10143864623
Accession Number: 3011620730

EXAM/PROCEDURE
Complete 2D/M-mode, color Doppler, and spectral Doppler echocardiogram performed. The study was technically adequate. No prior exam for comparison. Critical value: new LV systolic dysfunction, EF <35%. Sonographer communicated preliminary findings to Dr Damert Coronado at 09:44 on 01/13/2024.

Interpretation Summary
The left ventricle is severely dilated.
Global left ventricular systolic function is severely reduced.

LEFT VENTRICLE
The left ventricle is severely dilated.LVIDd = 6.2 cm. There is increased left ventricular mass with normal relative wall thickness consistent with severe eccentric hypertrophy. LV mass index = 168.0 grams/m^2. Relative wall thickness= 0.36 (nl <0.42).The ejection fraction = 26.2 % by biplane Method of Discs. Ejection Fraction = 25.5 %. Global longitudinal strain is -6.9%. Global left ventricular systolic function is severely reduced. There is no thrombus visualized. A false chord is noted (normal variant).

DIASTOLOGY/HEMODYNAMICS
There is Stage I impaired LV diastolic filling pattern. Avg. E/e' = 13.2 (nl <13). There is evidence for elevated left atrial pressure. Cardiac output is 3.4 l/min (nl 4-7l/min). Cardiac index is 1.9 l/min/m^2 (nl 2.5-4.5l/min/m2). Unable to estimate pulmonary arterial
pressure. Right atrial pressure is estimated as 15mmHg.

RIGHT VENTRICLE
The right ventricle is dilated. RVDd2 = 1.6 cm. RVDd1 = 4.6 cm. The right ventricular ejection fraction is mildly decreased. The right ventricular tricuspid annular plane systolic excursion (TAPSE) measures 1.6 cm.

ATRIA
The left atrium is mildly dilated. Left atrial volume (bi-plane) = 38.3 ml/m2 (nl < 35ml/m2). Right atrium is dilated. Right atrial volume indexed = 36.3 ml/m^2 (nl male/female <39/<34 ml/m^2). There is no Doppler evidence for a patent foramen ovale.

MITRAL VALVE
The mitral regurgitant jet is eccentrically directed. There is mild to moderate mitral regurgitation. Effective regurgitant orifice 0.13 cm^2 (severe >0.4cm2). Mitral regurgitation PISA = 0.50 cm. Regurgitant volume 20.9 ml (severe >/= 60ml/beat).

AORTIC VALVE
Normal trileaflet aortic valve. Trace aortic regurgitation.

TRICUSPID VALVE
The tricuspid valve is normal. There is physiologic tricuspid regurgitation.

PULMONIC VALVE
The pulmonic valve leaflets are thin and pliable; valve motion is normal. Trace pulmonic valvular regurgitation.

AORTA/PULMONARY ARTERY
The aortic sinus of valsalva is dilated. The Aortic Sinus(es) of Valsalva measures 3.8 cm. Indexed Aortic Sinus(es) of Valsalva are 2.2 cm/m2. (normal <=2.1cm/m2). The ascending aorta is dilated. The ascending aorta measures 3.7 cm. Indexed ascending aorta is 2.1 cm/m2. (normal <=1.9cm/m2). The pulmonary artery is dilated. The pulmonary artery measures 3.9 cm.

PERICARDIUM/PLEURAL
There is no pericardial effusion. Large right pleural effusion. The left lung base image was technically limited. A unilateral comet-tail artifact is visualized when imaging the anteroapex of the lung.

MMode/2D Measurements & Calculations
RVDd1(nl <4.2cm): 4.6 cm
RVDd2(nl <3.6cm): 1.6 cm
LVIDd (3.8-5.8): 6.2 cm
LVIDs (2.3-3.9): 5.4 cm

Impressions

IVSd (0.6-1.1): 1.1 cm
LVPWd (0.6-1.1): 1.1 cm

LV mass(C)d: 291.9 grams
LV mass idx (F<96,M<116g/m2): 168.0 grams/m2
Relative Wall Thickness: 0.36
Ao sinus diam: 3.8 cm
AO sinus indexed: 2.2 cm
asc Aorta Diam: 3.7 cm
asc AO indexed: 2.1 cm
LA dimension (1.9-4.0): 3.9 cm

LVOT diam: 1.8 cm

LVOT area: 2.5 cm2
LA area AP2: 26.1 cm2
LA area AP4: 18.3 cm2
LA shortest length: 6.1 cm
LA VOLUME (indexed): 38.3

MPA diam: 3.9 cm

EDV(MOD-bp): 127.4 ml

EDV(MOD-bp) indexed: 73.3 ml/m2
ESV(MOD-bp): 94.0 ml
EF(MOD-bp): 26.2 %
SV(MOD-bp): 33.3 ml

RA area AP4: 17.7 cm2
RA length: 5.0 cm
RA Volume Indexed: 36.3 ml/m2
TAPSE(nl >=1.7cm): 1.6 cm

Time Measurements
Aortic R-R: 0.82 sec
Aortic HR: 73.0 BPM

Doppler Measurements & Calculations
MV E max vel: 46.0 cm/sec
MV A max vel: 51.8 cm/sec
MV E/A: 0.89
MV dec slope: 206.0 cm/sec2
MV dec time: 0.22 sec
Lat Peak E' Vel: 3.5 cm/sec
Med Peak E' Vel: 3.5 cm/sec
Lateral E/e' ratio: 13.2
Average E/e' ratio: 13.2

Ao V2 max: 146.0 cm/sec
Ao max PG: 8.5 mmHg
Ao V2 mean: 99.4 cm/sec
Ao mean PG: 4.0 mmHg
Ao V2 VTI: 22.6 cm
AVA(I,D): 2.0 cm2
LV V1 max PG: 4.3 mmHg
LV V1 mean PG: 2.0 mmHg
LV V1 max: 104.0 cm/sec
LV V1 mean: 74.5 cm/sec
LV V1 VTI: 18.2 cm

AVA (I,D) indexed for BSA: 1.2 cm2
Dimensionless Index (LV/AV): 0.81

MR max vel: 451.0 cm/sec
MR max PG: 81.4 mmHg
MR mean vel: 355.0 cm/sec
MR mean PG: 55.0 mmHg
MR VTI: 156.0 cm
MR PISA radius: 0.50 cm
MR alias vel: 38.5 cm/sec
MR ERO: 0.13 cm2
MR volume: 20.9 ml
MR flow rate: 60.5 cm3/sec
CO(LVOT): 3.4 l/min
CI(LVOT): 1.9 l/min/m2
SV(LVOT): 46.3 ml
SI(LVOT): 26.7 ml/m2

PA V2 max: 73.0 cm/sec
PA max PG: 2.0 mmHg
RV V1 max: 56.1 cm/sec
RV V1 max PG: 1.3 mmHg
RV S'(nl>=9.5cm/sec): 12.4 cm/sec

MR regurgitant fraction: 31.1 %

Reading Physician:Electronically signed by: Arvin Narula, MD on 01/14/2024 07:37 AM
Ordering Physician: JONES, CHRISTOPHER
Performed By: Rodil, Alla, RDCS

Narrative

This result has an attachment that is not available.

Procedure Note

Arvin Narula, MD - 01/14/2024

Formatting of this note might be different from the original.

IMPRESSION:

Study ID: 1074047

Scripps Mercy Hospital Chula Vista
Cardiology Lab
435 H Street
Chula Vista, CA 91910
(619) 691-7499
Adult Echo Report
Name: PETRUCCIO, ELIZABETH TINA Study Date: 01/13/2024 08:39 AM
MRN: 810760324 Patient Location: SVICU^0ICU^11^CV
DOB: 07/29/1964 Height: 67 in
Age: 59 yrs Weight: 140 lb
Gender: Female BSA: 1.7 m2
HR: 66 BP: 105/83 mmHg
Reason For Study: Other Cardiac Reason, Specify in Comments
History: Heart failure reduced ejection fraction on echo, reported history of asthma; medical noncompliance, past alcohol and methamphetamine abuse
Acct Number: 10143864623 Accession Number: 3011620730

EXAM/PROCEDURE
Complete 2D/M-mode, color Doppler, and spectral Doppler echocardiogram performed. The study was technically adequate. No prior exam for comparison. Critical value: new LV systolic dysfunction, EF <35%. Sonographer communicated preliminary findings to Dr Damert Coronado at 09:44 on 01/13/2024.

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pressure. Right atrial pressure is estimated as 15mmHg.

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AORTIC VALVE
Normal trileaflet aortic valve. Trace aortic regurgitation.

TRICUSPID VALVE
The tricuspid valve is normal. There is physiologic tricuspid regurgitation.

PULMONIC VALVE
The pulmonic valve leaflets are thin and pliable; valve motion is normal. Trace pulmonic valvular regurgitation.

AORTA/PULMONARY ARTERY
The aortic sinus of valsalva is dilated. The Aortic Sinus(es) of Valsalva measures 3.8 cm. Indexed Aortic Sinus(es) of Valsalva are 2.2 cm/m2. (normal <=2.1cm/m2). The ascending aorta is dilated. The ascending aorta measures 3.7 cm. Indexed ascending aorta is 2.1 cm/m2. (normal <=1.9cm/m2). The pulmonary artery is dilated. The pulmonary artery measures 3.9 cm.

PERICARDIUM/PLEURAL
There is no pericardial effusion. Large right pleural effusion. The left lung base image was technically limited. A unilateral comet-tail artifact is visualized when imaging the anteroapex of the lung.

MMode/2D Measurements & Calculations
RVDd1(nl <4.2cm): 4.6 cm LVIDd (3.8-5.8): 6.2 cm
RVDd2(nl <3.6cm): 1.6 cm LVIDs (2.3-3.9): 5.4 cm
IVSd (0.6-1.1): 1.1 cm LVPWd (0.6-1.1): 1.1 cm

LV mass(C)d: 291.9 grams Ao sinus diam: 3.8 cm
LV mass idx (F<96,M<116g/m2): 168.0 grams/m2 AO sinus indexed: 2.2 cm
Relative Wall Thickness: 0.36 asc Aorta Diam: 3.7 cm
asc AO indexed: 2.1 cm
LA dimension (1.9-4.0): 3.9 cm

LVOT diam: 1.8 cm
LA area AP2: 26.1 cm2
LVOT area: 2.5 cm2 LA area AP4: 18.3 cm2
LA shortest length: 6.1 cm
LA VOLUME (indexed): 38.3

MPA diam: 3.9 cm
EDV(MOD-bp): 127.4 ml

EDV(MOD-bp) indexed: 73.3 ml/m2
ESV(MOD-bp): 94.0 ml
EF(MOD-bp): 26.2 %
SV(MOD-bp): 33.3 ml

RA area AP4: 17.7 cm2
RA length: 5.0 cm
RA Volume Indexed: 36.3 ml/m2
TAPSE(nl >=1.7cm): 1.6 cm

Time Measurements
Aortic R-R: 0.82 sec
Aortic HR: 73.0 BPM

Doppler Measurements & Calculations
MV E max vel: 46.0 cm/sec Lat Peak E' Vel: 3.5 cm/sec
MV A max vel: 51.8 cm/sec Med Peak E' Vel: 3.5 cm/sec
MV E/A: 0.89 Lateral E/e' ratio: 13.2
MV dec slope: 206.0 cm/sec2 Average E/e' ratio: 13.2
MV dec time: 0.22 sec

Ao V2 max: 146.0 cm/sec LV V1 max PG: 4.3 mmHg
Ao max PG: 8.5 mmHg LV V1 mean PG: 2.0 mmHg
Ao V2 mean: 99.4 cm/sec LV V1 max: 104.0 cm/sec
Ao mean PG: 4.0 mmHg LV V1 mean: 74.5 cm/sec
Ao V2 VTI: 22.6 cm LV V1 VTI: 18.2 cm
AVA(I,D): 2.0 cm2

AVA (I,D) indexed for BSA: 1.2 cm2
Dimensionless Index (LV/AV): 0.81

Procedure Note

MR max vel: 451.0 cm/sec CO(LVOT): 3.4 l/min
MR max PG: 81.4 mmHg CI(LVOT): 1.9 l/min/m2
MR mean vel: 355.0 cm/sec SV(LVOT): 46.3 ml
MR mean PG: 55.0 mmHg SI(LVOT): 26.7 ml/m2
MR VTI: 156.0 cm
MR PISA radius: 0.50 cm
MR alias vel: 38.5 cm/sec
MR ERO: 0.13 cm2
MR volume: 20.9 ml
MR flow rate: 60.5 cm3/sec

PA V2 max: 73.0 cm/sec RV V1 max: 56.1 cm/sec
PA max PG: 2.0 mmHg RV V1 max PG: 1.3 mmHg
RV S'(nl>=9.5cm/sec): 12.4 cm/sec

MR regurgitant fraction: 31.1 %

Reading Physician:Electronically signed by: Arvin Narula, MD on 01/14/2024 07:37 AM
Ordering Physician: JONES, CHRISTOPHER
Performed By: Rodil, Alla, RDCS

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	CV ECHO PROCEDURES	Edited Result - Final

BLOOD GAS ALL ANALYTES, ARTERIAL - Final result (01/13/2024 7:12 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.40	7.35 - 7.45		01/13/2024 7:16 AM PST	MERCY LAB CV	
pCO2, Arterial	42	35 - 45 mm Hg		01/13/2024 7:16 AM PST	MERCY LAB CV	
pO2, Arterial	286	83 - 108 mm Hg		01/13/2024 7:16 AM PST	MERCY LAB CV	
HCO3, Arterial	25.8	21.0 - 28.0 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
O2 Sat, Arterial	100.0	95.0 - 98.0 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	1.0	-2.0 - 3.0 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
FIO2, Arterial	100.0			01/13/2024 7:16 AM PST	MERCY LAB CV	
Oxygen Device	Ventilator			01/13/2024 7:16 AM PST	MERCY LAB CV	
Modes	PRVC			01/13/2024 7:16 AM PST	MERCY LAB CV	
Total Hgb, Whole Blood	11.1	11.9 - 15.3 g/dL		01/13/2024 7:16 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Oxyhemoglobin, Arterial	98.4	95.0 - 98.0 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Carboxyhemoglobin, Arterial	0.4	1.0 - 1.5 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Methemoglobin, Arterial	1.2	<3.0 %		01/13/2024 7:16 AM PST	MERCY LAB CV	
Lactic Acid, Arterial	1.1	0.4 - 0.9 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Glucose, Whole Blood	186	70 - 125 mg/dL		01/13/2024 7:16 AM PST	MERCY LAB CV	
Ionized Calcium, Whole Blood	1.17	1.15 - 1.27 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Sodium, Whole Blood	134	136 - 146 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Potassium, Whole Blood	5.3	3.5 - 5.1 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Chloride, Whole Blood	98	98 - 107 mmol/L		01/13/2024 7:16 AM PST	MERCY LAB CV	
Allen test	Positive			01/13/2024 7:16 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/13/2024 7:16 AM PST	MERCY LAB CV	
Comment	400/20/+8			01/13/2024 7:16 AM PST	MERCY LAB CV	
Operator ID	243,099			01/13/2024 7:16 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/13/2024 7:12 AM PST	01/13/2024 7:16 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/13/2024 6:59 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 1:42 PM PST	

Impressions

01/13/2024 1:43 PM PST
IMPRESSION: Endotracheal and gastric tubes as described below.

Unchanged large right pleural effusion

release

Narrative

01/13/2024 1:43 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 6:59 AM

HISTORY: intubation

COMPARISON: 1/13/2024

FINDINGS:

Endotracheal tube terminates 4.6 cm above the carina. Gastric tube terminates in the proximal stomach.

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis or infiltrate. No pneumothorax.

Imaged skeletal structures are unchanged.

Procedure Note

Raymond Hong, MD - 01/13/2024
Formatting of this note might be different from the original.
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 6:59 AM

HISTORY: intubation

COMPARISON: 1/13/2024

FINDINGS:

Endotracheal tube terminates 4.6 cm above the carina. Gastric tube terminates in the proximal stomach.

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis or infiltrate. No pneumothorax.

Imaged skeletal structures are unchanged.

IMPRESSION:
IMPRESSION: Endotracheal and gastric tubes as described below.

Unchanged large right pleural effusion

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

GLUCOSE, POC - Final result (01/13/2024 6:40 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	142	70 - 125 mg/dL		01/13/2024 6:42 AM PST	MERCY LAB CV	
Operator ID	157,150			01/13/2024 6:42 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 6:40 AM PST	01/13/2024 6:42 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CK - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
CK	148	30 - 135 Units/L		01/13/2024 5:55 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LIPID PANEL W/ REFLEX TO DIRECT LDL IF INDICATED - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/13/2024 1:02 PM PST	SCRIPPS LAB	
Triglycerides	72	7 - 150 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Comment:

```
Normal          <150      mg/dL
Borderline High  150-199  mg/dL
High             200-499  mg/dL
Very high       >=500     mg/dL

N-Acetyl-L-Cysteine at therapeutically achieved concentrations may lead to falsely low results.
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cholesterol	110	7 - 200 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Comment:

```
Desirable          <200        mg/dL
Borderline high     200-239    mg/dL
High               >= 240      mg/dL
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HDL Cholesterol	33	>=40 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Comment:

```
Major Risk Factor for Heart Disease
<40 mg/dL

Negative Risk Factor for Heart Disease
>= 60 mg/dL

N-acetyl-L-cysteine at elevated concentrations may lead to falsely low results
```

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
LDL Calculation	63	<100 mg/dL		01/13/2024 1:02 PM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

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SCRIPPS LAB - 01/13/2024 1:02 PM PST

Reference ranges are derived from the clinical practice ATPIII Guidelines of the National Cholesterol
Education Program (NCEP) on the Management of Blood Cholesterol: A Report of the American College of
Cardiology/American Heart Association Task Force on Clinical Practice Guidelines, More recent guidelines
using clinical and laboratory risk stratification approach provide more specific recommendations for
specific patient groups (November 2018).

The relationship between levels of LDL cholesterol and Coronary Heart Disease is continuous over a broad
range of LDL levels. A reduction in Cholesterol, Triglycerides and LDL levels has been associated with a
reduced risk of CHD.

https://www.ahajournals.org/doi/10.1161/CIR.0000000000000625
```

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

HIV ANTIGEN/ANTIBODY COMBO - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HIV Antigen / Antibody Combo	Non Reactive	Non Reactive		01/13/2024 12:02 PM PST	SCRIPPS LAB	

Comment: HIV-1 p24 antigen and HIV-1/HIV-2 antibodies are not detected.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:24 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

TROPONIN I - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.014	<=0.034 ng/mL		01/13/2024 6:00 AM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population. This patient's result is considered normal. However, clinical
correlation is suggested to determine whether serial measurements are necessary for further evaluation.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RENAL FUNCTION PANEL (RENAL PANEL) - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose	235	70 - 125 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Sodium	133	137 - 145 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Potassium	6.0	3.5 - 5.1 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Chloride	101	98 - 107 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
CO2, Total	28	22 - 30 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Urea Nitrogen	55	7 - 17 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Creatinine	1.39	0.50 - 1.00 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
CKD-EPI eGFR 2021	44	>60 mL/min/1.73m*2		01/13/2024 6:01 AM PST	MERCY LAB CV	
Anion Gap	4	6 - 14 mmol/L		01/13/2024 6:01 AM PST	MERCY LAB CV	
Osmolality Calculated	299	280 - 305 mOsm/kg H20		01/13/2024 6:01 AM PST	MERCY LAB CV	
Calcium	8.7	8.4 - 10.2 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Phosphorus	9.4	2.5 - 4.5 mg/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	
Albumin	3.4	3.5 - 5.0 g/dL		01/13/2024 6:01 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:23 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

HEMOGRAM - Final result (01/13/2024 5:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	21.4	3.4 - 11.0 K/mcL		01/13/2024 5:34 AM PST	MERCY LAB CV	
RBC	3.83	3.98 - 5.25 M/mcL		01/13/2024 5:34 AM PST	MERCY LAB CV	
HGB	11.8	11.9 - 15.3 g/dL		01/13/2024 5:34 AM PST	MERCY LAB CV	
HCT	38.8	37.3 - 46.7 %		01/13/2024 5:34 AM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		01/13/2024 5:34 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		01/13/2024 5:34 AM PST	MERCY LAB CV	
MCHC	30.4	31.0 - 36.0 g/dL		01/13/2024 5:34 AM PST	MERCY LAB CV	
RDW	14.9	11.7 - 14.9 %		01/13/2024 5:34 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Platelet Count	801	150 - 425 K/ mcL		01/13/2024 5:34 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/13/2024 5:14 AM PST	01/13/2024 5:24 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BLOOD GAS, ARTERIAL - Final result (01/13/2024 5:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.10	7.35 - 7.45		01/13/2024 5:09 AM PST	MERCY LAB CV	
pCO2, Arterial	87	35 - 45 mm Hg		01/13/2024 5:09 AM PST	MERCY LAB CV	
pO2, Arterial	117	83 - 108 mm Hg		01/13/2024 5:09 AM PST	MERCY LAB CV	
HCO3, Arterial	27.2	21.0 - 28.0 mmol/L		01/13/2024 5:09 AM PST	MERCY LAB CV	
O2 Sat, Arterial	97.0	95.0 - 98.0 %		01/13/2024 5:09 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	-2.4	-2.0 - 3.0 mmol/L		01/13/2024 5:09 AM PST	MERCY LAB CV	
FIO2, Arterial	60.0			01/13/2024 5:09 AM PST	MERCY LAB CV	
Oxygen Device	BIPAP			01/13/2024 5:09 AM PST	MERCY LAB CV	
Modes	NA			01/13/2024 5:09 AM PST	MERCY LAB CV	
Allen test	NA			01/13/2024 5:09 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/13/2024 5:09 AM PST	MERCY LAB CV	
Comment	14/7/16			01/13/2024 5:09 AM PST	MERCY LAB CV	
Notified By	243560			01/13/2024 5:09 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Notified Whom	157150			01/13/2024 5:09 AM PST	MERCY LAB CV	
Operator ID	243,560			01/13/2024 5:09 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/13/2024 5:01 AM PST	01/13/2024 5:09 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/13/2024 4:09 AM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 12:26 PM PST	

Impressions

01/13/2024 12:27 PM PST
IMPRESSION: No significant change.

release

Narrative

01/13/2024 12:27 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 4:09 AM

HISTORY: f/u thoracentesis

COMPARISON: 1/12/2024

FINDINGS:

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis. No pneumothorax.

Imaged skeletal structures are unchanged.

Procedure Note

Raymond Hong, MD - 01/13/2024

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/13/2024 4:09 AM

HISTORY: f/u thoracentesis

COMPARISON: 1/12/2024

FINDINGS:

The heart and mediastinum are unchanged.

Unchanged large right pleural effusion with right basilar atelectasis. No pneumothorax.

Imaged skeletal structures are unchanged.

IMPRESSION:
IMPRESSION: No significant change.

release

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

BLOOD GAS LACTIC ACID, ARTERIAL - Final result (01/13/2024 2:55 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.13	7.35 - 7.45		01/13/2024 3:11 AM PST	MERCY LAB CV	
pCO2, Arterial	84	35 - 45 mm Hg		01/13/2024 3:11 AM PST	MERCY LAB CV	
pO2, Arterial	78	83 - 108 mm Hg		01/13/2024 3:11 AM PST	MERCY LAB CV	
HCO3, Arterial	28.2	21.0 - 28.0 mmol/L		01/13/2024 3:11 AM PST	MERCY LAB CV	
O2 Sat, Arterial	90.5	95.0 - 98.0 %		01/13/2024 3:11 AM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	-1.0	-2.0 - 3.0 mmol/L		01/13/2024 3:11 AM PST	MERCY LAB CV	
FIO2, Arterial	32.0			01/13/2024 3:11 AM PST	MERCY LAB CV	
Oxygen Device	NC			01/13/2024 3:11 AM PST	MERCY LAB CV	
Modes	NA			01/13/2024 3:11 AM PST	MERCY LAB CV	
Lactic Acid, Arterial	1.1	0.4 - 0.9 mmol/L		01/13/2024 3:11 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Allen test	NA			01/13/2024 3:11 AM PST	MERCY LAB CV	
Site of draw	Brachial, left			01/13/2024 3:11 AM PST	MERCY LAB CV	
Comment	4L NC			01/13/2024 3:11 AM PST	MERCY LAB CV	
Notified By	243560			01/13/2024 3:11 AM PST	MERCY LAB CV	
Notified Whom	157150			01/13/2024 3:11 AM PST	MERCY LAB CV	
Operator ID	243,560			01/13/2024 3:11 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	01/13/2024 2:55 AM PST	01/13/2024 3:11 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PATHOLOGIST FLUID COMMENT - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Pathologist Review, CSF/ Body Fluids	Morphologic analysis of the smear shows predominantly acute inflammation. Yi Zhuang, M.D.			01/15/2024 9:24 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:07 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

NON-GYN CYTOPATHOLOGY - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Case Report	Cytopathology Report - Non-gyn Case: CC24-00016 Authorizing Provider: Jones, Christopher W, MD Collected: 01/13/2024 0202 Ordering Location: Scripps Mercy Hospital Received: 01/15/2024 1031 Chula Vista ICU Pathologist: Zhang, Sha, MD Specimen: Pleural cavity, right, body fluid			01/16/2024 2:12 PM PST	MERCY LAB CV	
Diagnosis	A: Pleural cavity, right, body fluid, cytology: Satisfactory for evaluation Negative for malignancy Acute inflammation and histiocytes			01/16/2024 2:12 PM PST	MERCY LAB CV	Electronically signed by Zhang, Sha, MD, PhD on 1/16/2024 at 1412 PST
Microscopic Examination	The microscopic findings support the above interpretation.			01/16/2024 2:12 PM PST	TORREY PINES LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Gross Examination	A Specimen received: volume: 2.5 cc, color: cloudy beige with debris. Specimen is forwarded to Green cytology for thinprep, cytospin, and cell block preparation. Mm TIF: 13:13 Slides prepared: 1 ThinPrep Pap stained monolayer, 1 Diff-Quik stained cytospin slide and 1 H&E stained cell block.			01/16/2024 2:12 PM PST	TORREY PINES LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Disclaimer	Processing and primary screening were performed at Scripps Medical Laboratories, Torrey Pines, 10666 N. Torrey Pines Road, La Jolla, CA 92037. Histology technical services were performed at Scripps Anatomic Pathology Laboratory, 9834 Genesee Avenue, La Jolla, CA 92037 Microscopic examination was performed at San Diego Pathologists Medical Group, 435 H Street, Chula Vista, CA 91910. If immunohistochemical staining test(s) were performed on this case at Scripps Anatomic Pathology Laboratory, they were developed and their performance characteristics determined by Scripps Anatomic Pathology Laboratory (9834 Genesee Avenue, La Jolla, CA 92037). It has not been cleared or approved by the U.S. Food and Drug Administration. The FDA has determined that such approval is not necessary. This test is used for clinical purposes. It should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA) as qualified to perform high complexity clinical laboratory testing. All controls show appropriate reactivity for each antibody reported.			01/16/2024 2:12 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Clinical History	Dx: Oral thrush Pleural effusion Prolonged QT interval COPD exacerbation (CMS/HCC) Acute kidney injury (CMS/HCC) Acute respiratory failure with hypercapnia (CMS/HCC) 59 yo F h/o HFrEF, Asthma, presents with shortness of breath, found to have concern for empyema			01/16/2024 2:12 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/15/2024 10:31 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB PATHOLOGY/ CYTOLOGY ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284
TORREY PINES LAB	10666 North Torrey Pines Rd	La Jolla, CA 92037	858-554-9560

CULTURE, WOUND (WOUND/FLUID/TISSUE CULTURE W/STAIN) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Wound Culture	No Growth Final		MIC	01/17/2024 8:53 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	Many WBCs			01/17/2024 8:53 AM PST	SCRIPPS LAB MICROBIOLOGY	
Gram Stain	No organisms seen			01/17/2024 8:53 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

ALBUMIN BODY FLUID (ALBUMIN, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Albumin (FL)	2.0	Not Established g/dL		01/13/2024 3:48 AM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/13/2024 3:48 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

MERCY LAB CV - 01/13/2024 3:48 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

GLUCOSE BODY FLUID (GLUCOSE, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/13/2024 3:48 AM PST	MERCY LAB CV	
Glucose (FL)	<20	Not Established mg/dL		01/13/2024 3:48 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

MERCY LAB CV - 01/13/2024 3:48 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LACTATE DEHYDROGENASE BODY FLUID (LACTATE DEHYDROGENASE, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/13/2024 4:06 AM PST	MERCY LAB CV	
Lactate Dehrdrogenase (FL)	1,379	Not Established Units/L		01/13/2024 4:06 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

MERCY LAB CV - 01/13/2024 4:06 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTEIN BODY FLUID (PROTEIN, BODY FLUID) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Source	Pleural cavity, right			01/13/2024 3:48 AM PST	MERCY LAB CV	
Protein (FL)	4.5	Not Established g/dL		01/13/2024 3:48 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:08 AM PST

Narrative

MERCY LAB CV - 01/13/2024 3:48 AM PST
This test was developed and its performance characteristics determined by Scripps Medical Laboratories. The test has not been cleared or approved by the U.S. Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BODY FLUID CELL COUNT WITH DIFF (BODY FLUID CELL COUNT WITH DIFFERENTIAL) - Final result (01/13/2024 2:02 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Volume (ML)	17.0	mL		01/13/2024 1:20 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fluid Color	Light Yellow			01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Appearance	Cloudy			01/13/2024 1:20 PM PST	MERCY LAB CV	
Comment: Cellular debris present may affect results.						
Fluid Nucleated Cell Count	103,752	/mcL		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Source	Pleural cavity, right			01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid RBC Count	20,000	/mcL		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Neutrophils %	94	%		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Lymphocytes %	1	%		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Mononuclears %	5	%		01/13/2024 1:20 PM PST	MERCY LAB CV	
Fluid Bacteria	Present	None Seen		01/13/2024 1:20 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:02 AM PST	01/13/2024 3:07 AM PST

Narrative

MERCY LAB CV - 01/13/2024 1:20 PM PST
Reference Interval has not been established for this specimen type.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BODY FLUIDS AND STOOLS ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

FUNGUS CULTURE - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Result	No Fungus Isolated Final		MIC	02/02/2024 7:51 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

AFB CULTURE W/STAIN - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
AFB Culture	No AFB isolated Final		MIC	03/09/2024 6:07 PM PST	SCRIPPS LAB MICROBIOLOGY	
AFB Smear	No AFB seen by fluorescent stain			03/09/2024 6:07 PM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Body Fluid	Structure of right pleural cavity / Unknown		01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

TRIGLYCERIDES BODY FLUID (TRIGLYCERIDES, BODY FLUIDS) - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Triglycerides Fluid Source	Pleural fluid			01/15/2024 11:53 AM PST	ARUP LAB	
Triglycerides (FL)	38	mg/dL		01/15/2024 11:53 AM PST	ARUP LAB	

Comment:

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INTERPRETIVE INFORMATION: Triglycerides, Fluid

For information on body fluid reference ranges and/or interpretive
guidance visit http://aruplab.com/bodyfluids/

This test was developed and its performance characteristics
determined by ARUP Laboratories. It has not been cleared or
approved by the US Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for
clinical purposes.
Performed By: ARUP Laboratories
500 Chipeta Way
Salt Lake City, UT 84108
Laboratory Director: Jonathan R. Genzen, MD, PhD
CLIA Number: 46D0523979
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Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Pleural Fluid			01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ARUP LAB	500 Chipeta Way	Salt Lake City, UT 84108-1221	800-522-2787

CHOLESTEROL,BODY FLUID (CHOLESTEROL, BODY FLUID) - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cholesterol Fluid Source	Pleural fluid			01/15/2024 11:53 AM PST	ARUP LAB	
Cholesterol (FL)	38	mg/dL		01/15/2024 11:53 AM PST	ARUP LAB	

Comment:

```
INTERPRETIVE INFORMATION: Cholesterol, Body Fluid

For information on body fluid reference ranges and/or interpretive
guidance visit http://aruplab.com/bodyfluids/

This test was developed and its performance characteristics
determined by ARUP Laboratories. It has not been cleared or
approved by the US Food and Drug Administration. This test was
performed in a CLIA certified laboratory and is intended for
clinical purposes.
Performed By: ARUP Laboratories
500 Chipeta Way
Salt Lake City, UT 84108
Laboratory Director: Jonathan R. Genzen, MD, PhD
CLIA Number: 46D0523979
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Pleural Fluid			01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ARUP LAB	500 Chipeta Way	Salt Lake City, UT 84108-1221	800-522-2787

SURVEILLANCE MRSA CULTURE - Final result (01/13/2024 2:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Staph Screen culture	NO Methicillin/ Oxacillin Resistant Staphylococcus aureus		MIC	01/14/2024 6:28 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasal structure / Unknown		01/13/2024 2:01 AM PST	01/13/2024 3:08 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

12-LEAD ECG - Final result (01/13/2024 12:14 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	118	bpm			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	92	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	294	ms			SCRIPPS RADIOLOGY	
QTC	412	ms			SCRIPPS RADIOLOGY	
R AXIS	60	degrees			SCRIPPS RADIOLOGY	
T AXIS	225	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/13/2024 12:14 AM PST	01/15/2024 8:57 AM PST

Impressions

SCRIPPS RADIOLOGY - 01/15/2024 8:57 AM PST
Atrial fibrillation with rapid ventricular response
Left ventricular hypertrophy with repolarization abnormality (Cornell product)

Narrative

This result has an attachment that is not available.

Procedure Note

Arvin Narula, MD - 01/15/2024
Formatting of this note might be different from the original.
IMPRESSION:
Atrial fibrillation with rapid ventricular response
Left ventricular hypertrophy with repolarization abnormality (Cornell product)

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

GLUCOSE, POC - Final result (01/13/2024 12:01 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose Blood, POC	121	70 - 125 mg/dL		01/13/2024 12:05 AM PST	MERCY LAB CV	
Operator ID	157,150			01/13/2024 12:05 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Capillary blood / Unknown		01/13/2024 12:01 AM PST	01/13/2024 12:05 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Pedro Dammert Coronado MD	LAB POCT ORDERABLES - DEVICE	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/12/2024 9:48 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/17/2024 11:01 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Structure of right hand / Unknown	Venipuncture / Unknown	01/12/2024 9:48 PM PST	01/12/2024 10:02 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CULTURE, BLOOD (BLOOD CULTURE) - Final result (01/12/2024 9:30 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Blood Culture	No Growth Final			01/17/2024 11:01 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Non-Lab Collection / Unknown	01/12/2024 9:30 PM PST	01/12/2024 10:03 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINE CULTURE - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Urine Culture	Multiple organisms present, probable skin/ urogenital contamination.		MIC	01/15/2024 8:35 AM PST	SCRIPPS LAB MICROBIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:35 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB MICROBIOLOGY	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

URINALYSIS MICROSCOPIC W/ REFLEX TO CULTURE - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC (UA)	10-20	0 - 2 / HPF		01/12/2024 9:43 PM PST	MERCY LAB CV	
RBC (UA)	6-10	0 - 2 / HPF		01/12/2024 9:43 PM PST	MERCY LAB CV	
Epithelial Cells, Squamous (UA)	Few	None / LPF		01/12/2024 9:43 PM PST	MERCY LAB CV	
Bacteria (UA)	Few	None / HPF		01/12/2024 9:43 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

FENTANYL, URINE - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fentanyl, Interpretation (Ur)	Negative	Negative		01/12/2024 9:43 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RAPID DRUG SCREEN, URINE (DRUG SCREEN, URINE) - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Benzodiazepines (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Cocaine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Tetrahydrocannabinol (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 50						
Opiates (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						
Methamphetamine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Barbiturates (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Amphetamine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Methadone (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Phencyclidine (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 25						
Tricyclic Antidepressants (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 300						
Oxycodone (UR)	Negative	Negative ng/mL		01/12/2024 9:48 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

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MERCY LAB CV - 01/12/2024 9:48 PM PST
This method provides screening results for medical purposes only. A more specific alternate chemical
method must be used for a confirmed analytical result. Clinical considerations and professional judgement
should be applied to any drug of abuse result, particularly when screening positive results are used.

Contact laboratory to order confirmatory testing.
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Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS, MACROSCOPIC, W/REFLEX TO MICROSCOPIC, W/REFLEX TO CULTURE (URINALYSIS, CULTURE IF INDICATED) - Final result (01/12/2024 9:03 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Color (UA)	Yellow	Yellow		01/12/2024 9:37 PM PST	MERCY LAB CV	
Clarity (UA)	Slightly Cloudy	Clear		01/12/2024 9:37 PM PST	MERCY LAB CV	
Specific Gravity (UA)	1.020	1.005 - 1.030		01/12/2024 9:37 PM PST	MERCY LAB CV	
pH (UA)	5.5	5.0 - 8.5 pH		01/12/2024 9:37 PM PST	MERCY LAB CV	
Protein (UA)	30	Negative mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	
Glucose (UA)	Negative	Negative mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	
Ketones (UA)	Negative	Negative mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	
Bilirubin (UA)	Negative	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Occult Blood (UA)	Moderate	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Leukocyte Esterase (UA)	Small	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Nitrite (UA)	Positive	Negative		01/12/2024 9:37 PM PST	MERCY LAB CV	
Urobilinogen (UA)	0.2	<2.0 mg/dL		01/12/2024 9:37 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen obtained by clean catch procedure / Unknown		01/12/2024 9:03 PM PST	01/12/2024 9:27 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (01/12/2024 8:49 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/12/2024 8:52 PM PST	

Impressions

01/12/2024 8:53 PM PST
IMPRESSION:
Large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly.

release

Narrative

01/12/2024 8:53 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/12/2024 8:49 PM

HISTORY: sepsis

COMPARISON: 12/22/2021

FINDINGS:
One view obtained.

Large right pleural effusion with subjacent atelectasis of the right lung. Left lung appears clear but appears hyperinflated. No pneumothorax.

The cardiomediastinal silhouette is enlarged. The osseous structures demonstrate no acute abnormality.

Procedure Note

Anshuman Bansal, MD - 01/12/2024

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 1/12/2024 8:49 PM

HISTORY: sepsis

COMPARISON: 12/22/2021

FINDINGS:
One view obtained.

Large right pleural effusion with subjacent atelectasis of the right lung. Left lung appears clear but appears hyperinflated. No pneumothorax.

The cardiomediastinal silhouette is enlarged. The osseous structures demonstrate no acute abnormality.

IMPRESSION:
IMPRESSION:
Large right pleural effusion with subjacent atelectasis. Hyperinflated left lung. Likely cardiomegaly.

release

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

BLOOD GAS LACTIC ACID, VENOUS - Final result (01/12/2024 7:44 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Venous	7.26	7.35 - 7.45		01/12/2024 7:53 PM PST	MERCY LAB CV	
pCO2, Venous	66	38 - 50 mm Hg		01/12/2024 7:53 PM PST	MERCY LAB CV	
pO2, Venous	28	Not Established mm Hg		01/12/2024 7:53 PM PST	MERCY LAB CV	
HCO3, Venous	29.5	22.0 - 29.0 mmol/L		01/12/2024 7:53 PM PST	MERCY LAB CV	
O2 Sat, Venous	41.8	60.0 - 85.0 %		01/12/2024 7:53 PM PST	MERCY LAB CV	
Base Excess/Deficit, Venous	2.3	Not Established mmol/L		01/12/2024 7:53 PM PST	MERCY LAB CV	
Lactic Acid, Venous	2.9	0.9 - 2.5 mmol/L		01/12/2024 7:53 PM PST	MERCY LAB CV	
Notification Note	CRITICAL VPH/VPCO2			01/12/2024 7:53 PM PST	MERCY LAB CV	
Notified By	128296			01/12/2024 7:53 PM PST	MERCY LAB CV	
Notified Whom	EMMA 241391 ER RN			01/12/2024 7:53 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Operator ID	128,296			01/12/2024 7:53 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:44 PM PST	01/12/2024 7:46 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROCALCITONIN - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Procalcitonin	0.29	0.03 - 0.08 ng/mL		01/13/2024 2:38 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

MERCY LAB CV - 01/13/2024 2:38 AM PST
Procalcitonin (PCT) is a dynamic biomarker with value in guiding antibiotic management in select clinical settings. PCT values are most useful when trends are analyzed. Decisions on antibiotic use should not be based solely on PCT level. Consider repeating PCT no more than daily in ICU/sepsis patients, and every 2 days for LRTI in order to discontinue antibiotics.

Suspected Lower Respiratory Tract Infection (LRTI):
0.1-0.25 ng/mL - low likelihood for bacterial infection, antibiotics discouraged
>0.25 ng/mL - increased likelihood for bacterial infection.

Suspected Sepsis
0.1-0.5 ng/mL - low likelihood for sepsis, antibiotics discouraged.
>0.5 ng/mL - increased likelihood of sepsis.
>2.0 ng/mL - high risk of sepsis/septic shock.

Consider discontinuation of antibiotics when PCT <0.25 (LRTI) or <0.5 (sepsis), or 80% reduction from baseline.

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

LIPASE - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Lipase	114	23 - 300 Units/L		01/13/2024 2:15 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Christopher Jones MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

MANUAL DIFFERENTIAL,REFLEXED FROM AUTO (MANUAL DIFFERENTIAL,REFLEXED) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Neutrophils %	93.0	38.0 - 74.0 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Bands %	3	0 - 6 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Lymphocytes %	2.0	16.0 - 48.0 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Monocytes %	2.0	4.9 - 12.5 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Eosinophils %	0.0	0.0 - 9.5 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Basophils %	0.0	0.0 - 1.6 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Neutrophils	17.39	1.50 - 7.40 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Bands	0.56	<=0.60 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Lymphocytes	0.37	0.90 - 3.10 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Monocytes	0.37	0.26 - 0.87 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Eosinophils	0.00	0.00 - 0.51 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
Absolute Basophils	0.00	0.00 - 0.09 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
PLT Slide Review	Verified by slide scan.			01/12/2024 9:40 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
RBC Slide Review	Reviewed			01/12/2024 9:40 PM PST	MERCY LAB CV	
Ovalocytes	Few			01/12/2024 9:40 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

THYROID STIMULATING HORMONE (ED) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Thyroid Stimulating Hormone	1.260	0.465 - 4.680 mcIU/mL		01/12/2024 8:31 PM PST	MERCY LAB CV	

Comment: High dose biotin supplementation (>1 mg/day) may falsely decrease results.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COVID-19, RAPID COMBO (COVID-19, RAPID (PCR/NAAT, COMBO)) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected		01/12/2024 8:53 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

A Detected result is considered a positive result for that virus.
A positive SARS-CoV-2 result indicates that RNA from SARS-CoV-2 was detected and that the patient is considered infected and presumed contagious. A positive Influenza A or B result indicates that RNA from Influenza A or B, respectively, was detected and that the patient is considered infected and presumed contagious.

A Not Detected (negative) test result means that RNA was not present in the specimen above the limit of detection. However, it does not rule out the possibility of infection and should not be used as the sole basis for patient management decisions. Test parameters for asymptomatic patients have not been evaluated.

These results have been reported to the San Diego Dept. of Health Services as required by Title 17, California Code of regulations, Section 2500.
Test information for patients can be found at the following link: https://www.fda.gov/media/142192/download
Test information for providers can be found at the following link: https://www.fda.gov/media/142191/download

This SARS-CoV-2 Real Time RT-PCR test was performed under an Emergency Use Authorization granted by the FDA for the Roche Liat.

This test meets Covid-19 testing requirements for school and business testing criteria and is not an antibody or antigen test.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

INFLUENZA A/B COMBO - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Influenza A PCR	Not Detected	Not Detected		01/12/2024 8:53 PM PST	MERCY LAB CV	
Influenza B PCR	Not Detected	Not Detected		01/12/2024 8:53 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

MERCY LAB CV - 01/12/2024 8:53 PM PST
These results have been reported to the San Diego Department of Health Services as required by Title 17, California Code of Regulations, Section 2505.

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	18.7	3.4 - 11.0 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
RBC	4.40	3.98 - 5.25 M/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
HGB	13.2	11.9 - 15.3 g/dL		01/12/2024 9:40 PM PST	MERCY LAB CV	
HCT	43.7	37.3 - 46.7 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
MCV	99	81 - 100 fL		01/12/2024 9:40 PM PST	MERCY LAB CV	
MCH	30	26 - 33 pg		01/12/2024 9:40 PM PST	MERCY LAB CV	
MCHC	30.2	31.0 - 36.0 g/dL		01/12/2024 9:40 PM PST	MERCY LAB CV	
RDW	14.8	11.7 - 14.9 %		01/12/2024 9:40 PM PST	MERCY LAB CV	
Platelet Count	799	150 - 425 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	
MPV	9.2	9.0 - 12.8 fL		01/12/2024 9:40 PM PST	MERCY LAB CV	
nRBC	0.0	0.0 /100 WBCs		01/12/2024 9:40 PM PST	MERCY LAB CV	
Preliminary Abs Neut	16.76	1.50 - 7.40 K/mcL		01/12/2024 9:40 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	13,700	Refer to the reference range comment. pg/mL		01/12/2024 8:12 PM PST	MERCY LAB CV	

Comment:

NT-proBNP reference range: <=125 pg/mL

NT-proBNP testing aids in the diagnosis or exclusion of heart failure (HF) when used in conjunction with clinical history, symptoms, exam findings, and additional imaging and laboratory studies.

Emergency Department:
The ICON-RELOADED study provided validation of cutoff values for NT-proBNP in patients presenting with dyspnea in the Emergency Department (1).
In this setting NT-proBNP level <300 pg/mL has a high negative predictive value for HF. For values above 300 pg/mL, age-specific cutoffs provide additional decision limit categories:

Result.....Age.....Interpretation
<300.....All.....Negative: Heart Failure Unlikely
=300 to <450.....22-<50..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <900.....50-<75..Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=300 to <1800....>=75....Gray Zone: Result Indeterminate-Consider other causes of NT-proBNP elevation*
=>450.....22-<50..Positive: Heart Failure Likely
=>900.....50-<75..Positive: Heart Failure Likely
=>1800.....>=75....Positive: Heart Failure Likely

* Natriuretic peptides values in the gray zones may be caused by conditions other than heart failure. Acute coronary syndrome,pulmonary embolism,pulmonary hypertension,sepsis,stroke,and renal dysfunction will elevate NT- proBNP levels. Obesity, flash pulmonary edema,cardiac tamponade,and pericardial constriction are conditions associated with reduced NT-proBNP(2).

Ambulatory Clinics:
For patients presenting to outpatient clinics with suspicion of HF, the optimal clinical utility of NT-proBNP is to
exclude heart failure.For this clinical setting Ortho Clinical Diagnostics recommends NT-proBNP results be interpreted
as in the table below(2):

Result.....Age.....Interpretation
<125.....All.....Negative: Heart Failure Unlikely
>=125.....All.....Consider Heart Failure as well as other causes* of NT-proBNP elevation.

* Natriuretic peptides value elevations could also be caused by conditions other than heart failure such as acute
coronary syndrome, pulmonary embolism, pulmonary hypertension, sepsis, stroke, and renal dysfunction (2).

References:
1. Terminal Pro-B-Type Natriuretic Peptide in the Emergency Department: The ICON-RELOADED Study. J Am Coll Cardiol. 2018 Mar 20;71(11):1191-1200. doi: 10.1016/j.jacc.2018.01.021. PMID: 29544601.

2. NBNP2 Instructions For Use. Ortho Clinical Diagnostics

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.029	<=0.034 ng/mL		01/12/2024 8:12 PM PST	MERCY LAB CV	

Comment:

```
The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit
in a healthy reference population. This patient's result is considered normal. However, clinical
correlation is suggested to determine whether serial measurements are necessary for further evaluation.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

APTT (ACTIVATED PTT) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
aPTT	31	Reference Range is 26-38. Heparin Therapeutic range is 53-87 Seconds		01/12/2024 8:04 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

PROTIME-INR (PROTHROMBIN TIME-INR) - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Protime	13.3	10.0 - 13.1 Seconds		01/12/2024 8:04 PM PST	MERCY LAB CV	
INR	1.1	Therapeutic Range: 2.0 - 3.0 Conventional Anticoagulation; 2.5 - 3.5 Intensive Anticoagulation		01/12/2024 8:04 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (01/12/2024 7:39 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				01/12/2024 8:01 PM PST	MERCY LAB CV	
Glucose	132	70 - 125 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Sodium	137	137 - 145 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Potassium	5.1	3.5 - 5.1 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Chloride	100	98 - 107 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
CO2, Total	32	22 - 30 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Urea Nitrogen	48	7 - 17 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Creatinine	1.09	0.50 - 1.00 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
CKD-EPI eGFR 2021	59	>60 mL/min/1.73m*2		01/12/2024 8:01 PM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		01/12/2024 8:01 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Osmolality Calculated	298	280 - 305 mOsm/kg H20		01/12/2024 8:01 PM PST	MERCY LAB CV	
Calcium	9.6	8.4 - 10.2 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Protein, total	8.8	6.3 - 8.2 g/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Albumin	3.7	3.5 - 5.0 g/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	
Alkaline Phosphatase	133	38 - 126 Units/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
AST	70	14 - 36 Units/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
ALT	31	<=34 Units/L		01/12/2024 8:01 PM PST	MERCY LAB CV	
Bilirubin, Total	0.8	0.2 - 1.3 mg/dL		01/12/2024 8:01 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	01/12/2024 7:39 PM PST	01/12/2024 7:45 PM PST

Narrative

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MERCY LAB CV - 01/12/2024 8:01 PM PST

for adults >19 years:
recumbent total protein values are approximately 0.4 dL less than ambulatory values.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (01/12/2024 7:36 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	152	bpm			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	98	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	318	ms			SCRIPPS RADIOLOGY	
QTC	505	ms			SCRIPPS RADIOLOGY	
R AXIS	69	degrees			SCRIPPS RADIOLOGY	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
T AXIS	212	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			01/12/2024 7:36 PM PST	01/15/2024 1:15 PM PST

Impressions

```
SCRIPPS RADIOLOGY - 01/15/2024 1:15 PM PST
** Critical Test Result: High HR
Atrial fibrillation with rapid ventricular response
Minimal voltage criteria for LVH, may be normal variant ( Cornell product )
Septal infarct , age undetermined
Abnormal ECG
When compared with ECG of 22-Dec-2021 20:06,
Atrial fibrillation has replaced Sinus rhythm
Septal infarct is now present
Nonspecific T wave abnormality, worse in Inferior leads
```

Narrative

This result has an attachment that is not available.

Procedure Note

Stephen Zanoni, MD - 01/15/2024

Formatting of this note might be different from the original.

IMPRESSION:
** Critical Test Result: High HR
Atrial fibrillation with rapid ventricular response
Minimal voltage criteria for LVH, may be normal variant (Cornell product)
Septal infarct , age undetermined
Abnormal ECG
When compared with ECG of 22-Dec-2021 20:06,
Atrial fibrillation has replaced Sinus rhythm
Septal infarct is now present
Nonspecific T wave abnormality, worse in Inferior leads

Authorizing Provider	Result Type	Result Status
Stephen Zanoni MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

Visit Diagnoses - documented in this encounter

Diagnosis
Respiratory insufficiency - Primary
COPD exacerbation
Fluid in pleural cavity
Oral yeast infection
Acute kidney injury
Prolonged Q-T interval on ECG
Loculated pleural effusion
Heart failure
Atrial fibrillation (irregular heartbeat)
Pus collection in space between lung and chest wall
Acute osteomyelitis of cervical spine

Admitting Diagnoses - documented in this encounter

Diagnosis
Respiratory insufficiency

Administered Medications - documented in this encounter

Inactive Administered Medications - up to 3 most recent administrations

Medication Order	MAR Action	Action Date	Dose	Rate	Site
acetaminophen (TYLENOL) suppository 650 mg 650 mg, rectal, Every 6 hours PRN, for, temperature greater than 38 C (100.4 F), Starting on Fri 1/12/24 at 2252, Do not exceed 4 g Acetaminophen per day `					
acetaminophen (TYLENOL) tablet 1,000 mg 1,000 mg, oral, Every 6 hours, First dose on Sat 1/13/24 at 0115, Do not exceed 4 g Acetaminophen per day `	Given	01/13/2024 1:05 AM PST	1,000 mg		
acetaminophen (TYLENOL) tablet 650 mg 650 mg, oral, Every 6 hours PRN, for, temperature greater than 38 C (100.4 F), Starting on Fri 1/12/24 at 2252, Do not exceed 4 g Acetaminophen per day `					
acetaminophen (TYLENOL) tablet 650 mg 650 mg, oral, Every 8 hours, First dose on Tue 1/16/24 at 0845, For 3 days, Do not exceed 4 g Acetaminophen per day `	Given Given Given	01/18/2024 5:29 PM PST 01/18/2024 7:58 AM PST 01/17/2024 5:30 PM PST	650 mg 650 mg 650 mg		
acyclovir (ZOVIRAX) 400 mg in dextrose 5 % (D5W) 278 mL IVPB 400 mg, intraVENOUS, at 278 mL/hr, Administer over 60 Minutes, Every 8 hours, First dose on Mon 1/15/24 at 1300, For 10 days, `Do Not Refrigerate, Indications: oral herpes	New Bag New Bag Rate/Dose Verify	01/16/2024 4:38 AM PST 01/15/2024 9:47 PM PST 01/15/2024 2:35 PM PST	400 mg 400 mg	278 mL/hr 278 mL/hr 278 mL/hr	
acyclovir (ZOVIRAX) 400 mg in dextrose 5 % (D5W) 278 mL IVPB 400 mg, intraVENOUS, at 278 mL/hr, Administer over 60 Minutes, Every 8 hours, First dose (after last modification) on Tue 1/16/24 at 1300, For 1 dose, `Do Not Refrigerate, Indications: oral herpes	New Bag	01/16/2024 1:30 PM PST	400 mg	278 mL/hr	
acyclovir (ZOVIRAX) suspension 400 mg 400 mg, oral, Every 8 hours scheduled, First dose on Tue 1/16/24 at 2100, For 9 days, SHAKE WELL BEFORE USE `Shake Well, Indications: HSV infection, treatment (non-CNS)	Given Given Given	01/25/2024 2:08 PM PST 01/25/2024 6:34 AM PST 01/24/2024 9:05 PM PST	400 mg 400 mg 400 mg		
albuterol 2.5 mg /3 mL (0.083 %) nebulizer solution 7.5 mg 7.5 mg, nebulization, Once, On Fri 1/12/24 at 1945, For 1 dose, `	Given	01/12/2024 8:02 PM PST	7.5 mg		
alteplase (CATHFLO ACTIVASE) 10 mg in sodium chloride 0.9 % (NS) 30 mL intrapleural syringe	Given	01/16/2024 3:54 PM PST	10 mg		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
10 mg, intraPLEURAL, Once, On Tue 1/16/24 at 1430, For 1 dose, Dispense this dose in a 60 mL syringe. To be administered by Physician/NP/PA only. Nurse to chart "Given by other". Clamp chest tube or catheter after administration. Allow 1 hour of dwell time. If possible and for best results, rotate patient to disperse medication. Unclamp chest tube or catheter and allow at least 2 hours of draining with or without suction. Contact Pharmacy 2 hours in advance of administration. Not to exceed 10 mg per dose. Room Temp, Syringe Type: Luer-Lok					
alteplase (CATHFLO ACTIVASE) 10 mg in sodium chloride 0.9 % (NS) 30 mL intrapleural syringe 10 mg, intraPLEURAL, Once, On Fri 1/19/24 at 1015, For 1 dose, Dispense this dose in a 60 mL syringe. To be administered by Physician/NP/PA only. Nurse to chart "Given by other". Clamp chest tube or catheter after administration. Allow 1 hour of dwell time. If possible and for best results, rotate patient to disperse medication. Unclamp chest tube or catheter and allow at least 2 hours of draining with or without suction. Contact Pharmacy 2 hours in advance of administration. Not to exceed 10 mg per dose. Room Temp, Syringe Type: Luer-Lok	Given	01/19/2024 11:49 AM PST	10 mg		
alum-mag hydroxide-simeth (MAALOX) 200-200-20 mg/5 mL suspension 30 mL 30 mL, oral, Every 6 hours PRN, indigestion / heartburn, Starting on Sat 1/13/24 at 0054, `Shake Well	Given	01/13/2024 1:07 AM PST	30 mL		
amiodarone (CORDARONE) 900 mg in dextrose 5 % (D5W) 500 mL (1.8 mg/mL) infusion 1 mg/min (33.3333 mL/hr, rounded to 33.3 mL/hr), intraVENOUS, Continuous, Starting on Sat 1/13/24 at 0130, For 6 hours, Infuse via central line whenever possible. Use 0.22 micron-inline filter if infusing via peripheral line Do not refrigerate. Protect from light. Use 0.22 micron in-line filter	Rate/Dose Verify Rate/Dose Verify New Bag	01/13/2024 5:00 AM PST 01/13/2024 2:22 AM PST 01/13/2024 2:17 AM PST	1 mg/min 1 mg/min 1 mg/min	33.3 mL/hr 33.3 mL/hr 33.3 mL/hr	
amiodarone (CORDARONE) 900 mg in dextrose 5 % (D5W) 500 mL (1.8 mg/mL) infusion 0.5 mg/min (16.6667 mL/hr, rounded to 16.7 mL/hr), intraVENOUS, Continuous, Starting on Sat 1/13/24 at 0817, Infuse via central line whenever possible. Use 0.22 micron-inline filter if infusing via peripheral line. Do not refrigerate. Protect from light. Use 0.22 micron in-line filter	Rate/Dose Verify Rate/Dose Verify Rate/Dose Verify	01/14/2024 7:00 AM PST 01/14/2024 2:00 AM PST 01/14/2024 12:00 AM PST	0.5 mg/min 0.5 mg/min 0.5 mg/min	16.7 mL/hr 16.7 mL/hr 16.7 mL/hr	
amiodarone (CORDARONE) IVPB 150 mg in dextrose 100 mL (premix) 150 mg, intraVENOUS, at 600 mL/hr, Administer over 10 Minutes, Once, On Sat 1/13/24 at 0130, For 1 dose, . Premix. Protect from light. Use 0.22 micron in-line filter	Rate/Dose Verify New Bag	01/13/2024 1:58 AM PST 01/13/2024 1:57 AM PST	 150 mg	600 mL/hr 600 mL/hr	
ammonium lactate (LAC-HYDRIN) 12 % lotion topical, 2 times daily, First dose on Tue 1/30/24 at 1300, Apply to the feet and hands `	Given Given Given	02/17/2024 8:25 AM PST 02/16/2024 9:05 PM PST 02/16/2024 9:22 AM PST			Specify Site Specify Site Right Foot
amoxicillin-clavulanate (AUGMENTIN) 875-125 mg per tablet 1 tablet	Given	01/25/2024 8:59 PM PST	1 tablet		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
1 tablet, oral, 2 times daily, First dose on Mon 1/22/24 at 2215, For 28 days, `, Indications: pulmonary infection	Given Given	01/25/2024 8:33 AM PST 01/24/2024 9:05 PM PST	1 tablet 1 tablet		
apixaban (ELIQUIS) tablet 5 mg 5 mg, oral, 2 times daily, First dose on Sat 2/3/24 at 2115, May crush and suspend tablets in 60 mL of water or D5W for nasogastric tube administration; administer immediately after preparation `, Indications: Non-Valvular Atrial Fibrillation	Given Given Given	02/17/2024 8:23 AM PST 02/16/2024 9:04 PM PST 02/16/2024 9:20 AM PST	5 mg 5 mg 5 mg		
artificial tears ophthalmic solution 1 drop 1 drop, each eye, 4 times daily PRN, dry eyes, Starting on Sun 2/4/24 at 1857	Given	02/04/2024 9:16 PM PST	1 drop		
azithromycin (ZITHROMAX) 500 mg in sodium chloride 0.9 % (NS) 250 mL Vial2Bag 500 mg, intraVENOUS, at 270 mL/hr, Administer over 60 Minutes, Once, On Fri 1/12/24 at 2130, For 1 dose, `Vial-2-Bag medication. Using V2B adapter and diluent bag, assemble and mix before use. For medication administration, scan the vial barcode, scan the pump, then scan the diluent bag. Ensure you have selected the correct diluent bag size for this order., Indications: pulmonary infection	Restarted New Bag	01/13/2024 1:05 AM PST 01/12/2024 11:58 PM PST	 500 mg	270 mL/hr 270 mL/hr	
benzonatate (TESSALON) capsule 100 mg 100 mg, oral, 3 times daily PRN, cough, Starting on Sat 1/20/24 at 1823, `Do not crush, chew, break or open. Swallow whole.	Given Given Given	01/26/2024 6:53 PM PST 01/22/2024 8:46 AM PST 01/20/2024 8:51 PM PST	100 mg 100 mg 100 mg		
bisaCODYL (DULCOLAX) suppository 10 mg 10 mg, rectal, Daily PRN, constipation, Starting on Fri 1/12/24 at 2250, Use in addition to the scheduled bowel medications if two days without bowel movement after initiation. Continue until bowel movement. `					
budesonide (PULMICORT) 0.25 mg/2 mL nebulizer solution 0.25 mg 0.25 mg, nebulization, Once PRN, administer with SOB, and wheezing, Starting on Sat 1/20/24 at 1213, For 1 dose, Rinse mouth with water after use to reduce candidiasis incidence . Do not swallow. `	Given	01/20/2024 12:54 PM PST	0.25 mg		
calcium carbonate (TUMS) chewable tablet 500 mg 500 mg, oral, 2 times daily PRN, indigestion / heartburn, Starting on Sun 1/21/24 at 1111, Chew tablet `	Given Given Given	02/02/2024 10:51 PM PST 01/22/2024 10:01 AM PST 01/21/2024 5:21 PM PST	500 mg 500 mg 500 mg		
carvediloL (COREG) tablet 3.125 mg 3.125 mg, oral, 2 times daily, First dose on Sat 1/27/24 at 0900, Hold for SBP less than 100. `	Given Given Given	02/17/2024 8:23 AM PST 02/16/2024 9:04 PM PST 02/16/2024 9:20 AM PST	3.125 mg 3.125 mg 3.125 mg		
cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g	Given	01/21/2024 9:33 PM PST	2 g	120 mL/hr	

Medication Order	MAR Action	Action Date	Dose	Rate	Site
2 g, intraVENOUS, at 120 mL/hr, Every 24 hours, First dose on Fri 1/12/24 at 2215, For 13 doses, `Refrigerate. Administer IV Push each 1 gram over 5 minutes., Indications: pneumonia	Given	01/20/2024 8:51 PM PST	2 g	120 mL/hr	
	Given	01/19/2024 10:42 PM PST	2 g	120 mL/hr	
cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g 2 g, intraVENOUS, at 120 mL/hr, Every 24 hours, First dose on Thu 1/25/24 at 2145, For 7 days, Refrigerate. Administer IV push up to 2 g over 5 minutes., Indications: Osteomyelitis, discitis	Given	01/29/2024 9:03 PM PST	2 g	120 mL/hr	
	Given	01/28/2024 8:52 PM PST	2 g	120 mL/hr	
	Given	01/27/2024 8:57 PM PST	2 g	120 mL/hr	
cefTRIAXone (ROCEPHIN) in sterile water IV Syringe 2 g 2 g, intraVENOUS, at 120 mL/hr, Every 24 hours, First dose (after last modification) on Tue 1/30/24 at 1600, For 52 days, Refrigerate. Administer IV push up to 2 g over 5 minutes., Indications: Osteomyelitis, discitis	Given	02/17/2024 3:47 PM PST	2 g	120 mL/hr	
	Given	02/16/2024 3:32 PM PST	2 g	120 mL/hr	
	Given	02/15/2024 4:08 PM PST	2 g	120 mL/hr	
chlorhexidine (PERIDEX) 0.12 % solution 15 mL 15 mL, mouth/throat, 2 times daily (Respiratory), First dose on Sat 1/13/24 at 0700, Discontinue when patient is extubated `	Given	01/14/2024 7:30 AM PST	15 mL		
	Given	01/13/2024 7:10 PM PST	15 mL		
	Given	01/13/2024 7:25 AM PST	15 mL		
clotrimazole (MYCELEX) troche 10 mg 10 mg, mucous membrane, 3 times daily, First dose on Tue 1/16/24 at 1500, Allow troche to dissolve slowly in the mouth `	Given	01/22/2024 8:48 PM PST	10 mg		
	Given	01/22/2024 3:10 PM PST	10 mg		
	Given	01/22/2024 8:47 AM PST	10 mg		
clotrimazole (MYCELEX) troche 10 mg 10 mg, mucous membrane, 3 times daily, First dose (after last modification) on Tue 1/23/24 at 0900, For 10 days, Allow troche to dissolve slowly in the mouth `	Given	01/25/2024 8:59 PM PST	10 mg		
	Given	01/25/2024 2:08 PM PST	10 mg		
	Given	01/25/2024 8:33 AM PST	10 mg		
colchicine (COLCRYS) tablet 0.6 mg 0.6 mg, oral, Daily, First dose on Fri 1/19/24 at 2145, `HAZARD LEVEL LOW special handling required; refer to policy.	Given	01/21/2024 8:07 AM PST	0.6 mg		
	Given	01/20/2024 8:45 AM PST	0.6 mg		
	Given	01/19/2024 10:42 PM PST	0.6 mg		
colchicine (COLCRYS) tablet 0.6 mg 0.6 mg, oral, Daily, First dose (after last modification) on Wed 1/24/24 at 0900, `HAZARD LEVEL LOW special handling required; refer to policy.	Given	01/25/2024 8:33 AM PST	0.6 mg		
	Given	01/24/2024 8:57 AM PST	0.6 mg		
colchicine (COLCRYS) tablet 0.6 mg 0.6 mg, oral, Daily, First dose on Fri 2/9/24 at 1345, `HAZARD LEVEL LOW special handling required; refer to policy.	Given	02/17/2024 8:23 AM PST	0.6 mg		
	Given	02/16/2024 9:19 AM PST	0.6 mg		
	Given	02/15/2024 8:24 AM PST	0.6 mg		
colchicine (COLCRYS) tablet 1.2 mg	Given	01/23/2024 8:16 AM PST	1.2 mg		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
1.2 mg, oral, Daily, First dose (after last modification) on Mon 1/22/24 at 0900, `HAZARD LEVEL LOW special handling required; refer to policy.	Given	01/22/2024 8:46 AM PST	1.2 mg		
dapagliflozin propanediol (FARXIGA) tablet 5 mg 5 mg, oral, Daily, First dose on Fri 2/9/24 at 1045, Administer in the morning with or without food. Hold at least 3 days prior to scheduled surgery. `	Given Given Given	02/17/2024 8:23 AM PST 02/16/2024 9:20 AM PST 02/15/2024 8:25 AM PST	5 mg 5 mg 5 mg		
dexamethasone (DECADRON) injection 10 mg 10 mg, intraVENOUS, Once, On Fri 1/12/24 at 1945, For 1 dose, `	Given	01/12/2024 7:50 PM PST	10 mg		
dexmedetomidine (PRECEDEX) 400 mcg in 0.9% NaCl 100 mL (4 mcg/mL) infusion (premix) 0-0.7 mcg/kg/hr × 65.1 kg (0-11.3925 mL/hr, rounded to 0-11.4 mL/hr), intraVENOUS, Titrated, Starting on Sat 1/13/24 at 0230, Start at 0.2 mcg/kg/hr. Titrate no faster than 0.2 mcg/kg/hr every 15 minutes until ordered Target Arousal level achieved. When sedation goal has been met for at least 12 consecutive hours then once per shift, attempt to decrease Dexmedetomidine (PRECEDEX) infusion while maintaining sedation goal. `Do not refrigerate. Premix., Titration Indication: Sedation	New Bag	01/13/2024 2:42 AM PST	0.2 mcg/kg/hr	3.3 mL/hr	
dextrose 5 % and lactated Ringer's (D5LR) infusion 30 mL/hr, intraVENOUS, Continuous, Starting on Sat 1/13/24 at 0230, For 1 day 6 hours, `Premix	Rate/Dose Verify Rate/Dose Verify New Bag	01/14/2024 7:00 AM PST 01/14/2024 2:00 AM PST 01/14/2024 1:15 AM PST	 30 mL/hr	30 mL/hr 30 mL/hr 30 mL/hr	
dextrose 5 % and sodium chloride 0.45 % (D5W 1/2 NS) infusion 75 mL/hr, intraVENOUS, Continuous PRN, ., Starting on Sat 1/13/24 at 0846, Start if patient is expected to miss more than 2 meals and/or changed to NPO. Discontinue when patient resumes eating and/or is no longer NPO. `Premix					
dilTIAZem (CarDIZem) injection 10 mg 10 mg, intraVENOUS, Administer over 2-5 Minutes, Once, On Fri 1/12/24 at 2000, For 1 dose, `Refrigerate	Given	01/12/2024 7:50 PM PST	10 mg		
dilTIAZem in dextrose 5 % (CarDIZem) 125 mg/125 mL (1 mg/mL) Premix solution 10 mg/hr (10 mL/hr), intraVENOUS, Continuous, Starting on Fri 1/12/24 at 2000, . `Premix. Use dedicated line	New Bag	01/12/2024 7:59 PM PST	10 mg/hr	10 mL/hr	
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 15 mL 15 mL, mouth/throat, 4 times daily before meals and at bedtime, First dose on Sun 1/14/24 at 1345, Each 15 mL contains diphenhydrAMINE 12.5 mg, lidocaine 100 mg, and aluminum hydroxide-magnesium hydroxide-simethicone 200-200-20 mg `Room Temperature	Given Given Given	02/17/2024 11:46 AM PST 02/17/2024 6:45 AM PST 02/16/2024 9:05 PM PST	15 mL 15 mL 15 mL		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
diphenhydrAMINE-lidocaine viscous-MAALOX 1:1:1 (BLM) mouthwash 30 mL 30 mL, mouth/throat, Once, On Fri 1/12/24 at 2015, For 1 dose, Each 15 mL contains diphenhydrAMINE 12.5 mg, lidocaine 100 mg, and aluminum hydroxide-magnesium hydroxide-simethicone 200-200-20 mg `Room Temperature	Given	01/12/2024 8:15 PM PST	30 mL		
dornase alfa (PULMOZYME) 5 mg in sterile water (SWFI) 30 mL 5 mg, intraPLEURAL, Once, On Tue 1/16/24 at 1430, For 1 dose, Dispense this dose in a 60 mL syringe. To be administered by Physician/NP/PA only. Nurse to chart "Given by other". Flush with NS 20 mL after administration. Clamp chest tube or catheter after administration. Allow 1 hour of dwell time. If possible and for best results, rotate patient to disperse medication. Unclamp chest tube or catheter and allow at least 2 hours of draining with or without suction. Contact Pharmacy 2 hours in advance of administration. Not to exceed 5 mg per dose. `Room temp. Protect from light., Syringe Type: Luer-Lok	Given	01/16/2024 3:54 PM PST	5 mg		
dornase alfa (PULMOZYME) 5 mg in sterile water (SWFI) 30 mL 5 mg, intraPLEURAL, Once, On Fri 1/19/24 at 1015, For 1 dose, Dispense this dose in a 60 mL syringe. To be administered by Physician/NP/PA only. Nurse to chart "Given by other". Flush with NS 20 mL after administration. Clamp chest tube or catheter after administration. Allow 1 hour of dwell time. If possible and for best results, rotate patient to disperse medication. Unclamp chest tube or catheter and allow at least 2 hours of draining with or without suction. Contact Pharmacy 2 hours in advance of administration. Not to exceed 5 mg per dose. `Room temp. Protect from light., Syringe Type: Luer-Lok	Given	01/19/2024 11:49 AM PST	5 mg		
enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg 40 mg, subcutaneous, Daily, First dose (after last modification) on Tue 1/16/24 at 1400, `	Given Given Given	02/03/2024 8:08 AM PST 02/02/2024 9:26 AM PST 02/01/2024 9:01 AM PST	40 mg 40 mg 40 mg		Left Lower Abdomen Left Lower Abdomen Left Lower Abdomen
etomidate (AMIDATE) injection 20 mg 20 mg, intraVENOUS, Once, On Sat 1/13/24 at 0545, For 1 dose, `	Given	01/13/2024 5:45 AM PST	20 mg		
fentaNYL (SUBLIMAZE) bolus from bag 25 mcg 25 mcg, intraVENOUS, Every 5 min PRN, for, to achieve analgesia goal, Starting on Sat 1/13/24 at 0515, Analgesia Goal - Critical Care Pain Observation Tool (CPOT): 0 `	Bolus from Bag Bolus from Bag Bolus from Bag	01/13/2024 2:35 PM PST 01/13/2024 11:15 AM PST 01/13/2024 11:10 AM PST	25 mcg 25 mcg 25 mcg		
fentaNYL citrate (PF) (SUBLIMAZE) 50 mcg/mL infusion	Rate/Dose Verify Rate/Dose Verify Rate/Dose Verify	01/14/2024 7:00 AM PST 01/14/2024 2:00 AM PST 01/14/2024 12:00 AM PST	50 mcg/hr 50 mcg/hr 50 mcg/hr	1 mL/hr 1 mL/hr 1 mL/hr	

Medication Order	MAR Action	Action Date	Dose	Rate	Site
0-200 mcg/hr (0-4 mL/hr), intraVENOUS, Titrated, Starting on Sat 1/13/24 at 0545, Start at 15 mcg/hr. If patient is requiring greater than 2 bolus doses per hour to achieve analgesia goal, may titrate no faster than 25 mcg/hr every 15 minutes to achieve goal. Critical Care Pain Observation Tool (CPOT) score: 0. Do NOT Refrigerate. LATEX FREE. Premix					
fluconazole in NaCl (iso-osm) (DIFLUCAN) IVPB 200 mg 200 mg, intraVENOUS, at 100 mL/hr, Administer over 60 Minutes, Once, On Sat 1/13/24 at 0230, For 1 dose, Do NOT Refrigerate. Premix. HAZARD LEVEL LOW special handling required; refer to policy., Indications: oral thrush, patient on bipap	New Bag	01/13/2024 3:11 AM PST	200 mg	100 mL/hr	
furosemide (LASIX) injection 40 mg 40 mg, intraVENOUS, Once, On Sun 1/14/24 at 0830, For 1 dose, `Do not refrigerate	Given	01/14/2024 8:35 AM PST	40 mg		
furosemide (LASIX) injection 40 mg 40 mg, intraVENOUS, Daily, First dose on Sat 1/27/24 at 1245, For 3 doses, `Do not refrigerate	Given Given Given	01/29/2024 8:46 AM PST 01/28/2024 9:02 AM PST 01/27/2024 1:38 PM PST	40 mg 40 mg 40 mg		
furosemide (LASIX) injection 40 mg 40 mg, intraVENOUS, Once, On Tue 2/6/24 at 1315, For 1 dose, `Do not refrigerate	Given	02/06/2024 1:43 PM PST	40 mg		
furosemide (LASIX) injection 40 mg 40 mg, intraVENOUS, Daily, First dose on Thu 2/8/24 at 0900, For 3 doses, `Do not refrigerate	Given Given Given	02/10/2024 9:23 AM PST 02/09/2024 8:57 AM PST 02/08/2024 10:09 AM PST	40 mg 40 mg 40 mg		
furosemide (LASIX) tablet 20 mg 20 mg, oral, Daily, First dose on Sat 1/20/24 at 1000, `	Given Given Given	02/06/2024 8:24 AM PST 02/05/2024 8:48 AM PST 02/01/2024 9:08 AM PST	20 mg 20 mg 20 mg		
furosemide (LASIX) tablet 40 mg 40 mg, oral, Daily, First dose (after last modification) on Wed 2/7/24 at 0900, `, On hold since Wed 2/7/2024 at 1228 until manually unheld	Given	02/07/2024 9:10 AM PST	40 mg		
furosemide (LASIX) tablet 40 mg 40 mg, oral, Daily, First dose on Sun 2/11/24 at 0900, `	Given Given Given	02/17/2024 8:23 AM PST 02/16/2024 9:20 AM PST 02/15/2024 8:24 AM PST	40 mg 40 mg 40 mg		
gabapentin (NEURONTIN) capsule 100 mg 100 mg, oral, 3 times daily, First dose on Tue 2/6/24 at 1500, `	Given Given Given	02/11/2024 8:12 AM PST 02/10/2024 9:55 PM PST 02/10/2024 4:14 PM PST	100 mg 100 mg 100 mg		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
gabapentin (NEURONTIN) capsule 200 mg 200 mg, oral, 3 times daily, First dose (after last modification) on Sun 2/11/24 at 1500, `	Given Given Given	02/17/2024 3:45 PM PST 02/17/2024 8:23 AM PST 02/16/2024 9:04 PM PST	200 mg 200 mg 200 mg		
gadoterate meglumine (DOTAREM) injection 14 mL 14 mL (rounded from 13.6 mL = 0.2 mL/kg × 68 kg), intraVENOUS, Once in imaging, contrast, Starting on Fri 1/26/24 at 0942, For 1 dose, Do not administer if patient does not meet inclusion criteria or does meet any exclusion criteria. `	Given	01/26/2024 10:08 AM PST	14 mL		Right Anticubital
heparin (porcine) 5,000 unit/mL injection 5,000 Units 5,000 Units, subcutaneous, Every 8 hours scheduled, First dose on Fri 1/12/24 at 2315, `	Given Given Given	01/16/2024 6:41 AM PST 01/15/2024 10:15 PM PST 01/15/2024 1:46 PM PST	5,000 Units 5,000 Units 5,000 Units		Left Lower Abdomen Left Lower Abdomen Left Lower Abdomen
heparin PF 1000 unit/NS 500 mL BAG Continuous PRN, Starting on Tue 1/30/24 at 1548, Intraprocedure (IR)	New Bag	01/30/2024 3:48 PM PST	500 mL		
HYDROcodone-acetaminophen (NORCO) 10-325 mg per tablet 1 tablet 1 tablet, oral, Every 4 hours PRN, severe pain (level 7-10), Starting on Wed 2/7/24 at 1104, Do not exceed 4 g Acetaminophen per day `	Given Given Given	02/10/2024 9:23 AM PST 02/10/2024 2:29 AM PST 02/09/2024 2:32 PM PST	1 tablet 1 tablet 1 tablet		
HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.2 mg 0.2 mg, intraVENOUS, Once, On Sat 1/13/24 at 0245, For 1 dose, `	Given	01/13/2024 2:38 AM PST	0.2 mg		
HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.25 mg 0.25 mg, intraVENOUS, Every 4 hours PRN, severe pain (level 7-10), Starting on Sat 2/10/24 at 1302, `	Given Given Given	02/16/2024 4:57 AM PST 02/15/2024 1:56 AM PST 02/14/2024 3:38 PM PST	0.25 mg 0.25 mg 0.25 mg		
HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg 0.5 mg, intraVENOUS, Every 3 hours PRN, moderate pain (level 4-6), severe pain (level 7-10), Starting on Sun 1/14/24 at 1135, `	Given Given Given	01/17/2024 6:58 AM PST 01/16/2024 9:45 PM PST 01/16/2024 3:44 PM PST	0.5 mg 0.5 mg 0.5 mg		
HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg 0.5 mg, intraVENOUS, Every 3 hours PRN, severe pain (level 7-10), Starting on Wed 1/17/24 at 1147, `	Given Given Given	02/07/2024 10:27 AM PST 02/07/2024 1:07 AM PST 02/06/2024 4:35 PM PST	0.5 mg 0.5 mg 0.5 mg		
HYDROmorphone (DILAUDID) 0.5 mg/0.5 mL injection syringe 0.5 mg 0.5 mg, intraVENOUS, Once, On Fri 1/19/24 at 1430, For 1 dose, `	Given	01/19/2024 2:18 PM PST	0.5 mg		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
hydrOXYzine pamoate (VISTARIL) capsule 25 mg 25 mg, oral, 3 times daily PRN, anxiety, Starting on Sun 1/21/24 at 1247, `	Given Given Given	02/12/2024 4:10 PM PST 02/06/2024 2:39 AM PST 02/04/2024 11:50 PM PST	25 mg 25 mg 25 mg		
iohexol (OMNIPAQUE 350) 350 mg iodine/mL solution 100 mL 100 mL, intraVENOUS, Once in imaging, contrast, Starting on Mon 1/29/24 at 1505, For 1 dose, Do not administer if patient does not meet all inclusion criteria or meets any exclusion criteria. `	Given	01/29/2024 3:11 PM PST	100 mL		
ipratropium (ATROVENT) 0.02 % nebulizer solution 0.5 mg 0.5 mg, nebulization, Once, On Fri 1/12/24 at 1945, For 1 dose, `	Given	01/12/2024 8:02 PM PST	0.5 mg		
ipratropium-albuterol (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL 3 mL, nebulization, Every 4 hours PRN, wheezing, Starting on Mon 1/15/24 at 0015, `	Given Given Given	01/19/2024 3:51 PM PST 01/18/2024 8:36 PM PST 01/18/2024 1:55 PM PST	3 mL 3 mL 3 mL		
ipratropium-albuterol (DUONEB) 0.5-2.5 (3) mg/3 mL nebulizer solution 3 mL 3 mL, nebulization, Every 4 hours PRN, wheezing, shortness of breath, Starting on Sat 1/20/24 at 0029, `	Given Given Given	02/16/2024 12:54 AM PST 02/14/2024 5:59 PM PST 02/14/2024 12:53 AM PST	3 mL 3 mL 3 mL		
Kit for Preparation of technetium Tc-99m mebrofenin injection 5.3 millicurie 5.3 millicurie, intraVENOUS, Once, On Fri 2/16/24 at 1415, For 1 dose, `Radiopharmaceutical: Use appropriate precautions for handling, disposal, and minimizing exposure to patients and healthcare personnel.	Given	02/16/2024 2:15 PM PST	5.3 millicuries		
lactulose (ENULOSE) 20 gram/30 mL solution 20 g 20 g, oral, 2 times daily PRN, for, no bowel movement for 3 days, Starting on Fri 1/12/24 at 2250, Use in addition to the scheduled medications and bisacodyl (DULCOLAX) if three days without bowel movement after initiation. Continue until bowel movement. Hold for nausea. `					
lidocaine (XYLOCAINE) 20 mg/mL (2 %) vial 5 mL 5 mL, injection, Once, On Sun 1/21/24 at 1930, For 1 dose, `	Given by Other	01/21/2024 7:15 PM PST	5 mL		Specify Site
lidocaine (XYLOCAINE) 20 mg/mL (2 %) vial As needed, Starting on Tue 1/30/24 at 1547, Intraprocedure (IR)	Given	01/30/2024 3:47 PM PST	5 mL		
lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL 1 mL (1 spray), mucous membrane, As needed, mouth sore, Starting on Mon 1/15/24 at 0341, `	Given	01/15/2024 4:18 AM PST	1 mL		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
lidocaine (XYLOCAINE) 4 % (40 mg/mL) topical solution 1 mL 1 mL (1 spray), mucous membrane, 4 times daily PRN, mouth sore, Starting on Mon 1/15/24 at 0430, `	Given Given Given	01/23/2024 8:24 PM PST 01/21/2024 2:28 PM PST 01/21/2024 3:49 AM PST	1 mL 1 mL 1 mL		
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch 1 patch, transdermal, Administer over 12 Hours, Every 24 hours, First dose on Mon 1/15/24 at 0445, Apply to left and right neck for 12 hours,remove for 12 hours `Patch may be cut. Remove old patch	Medication Applied Medication Applied Medication Applied	02/11/2024 5:10 AM PST 02/10/2024 5:23 AM PST 02/09/2024 4:34 AM PST	1 patch 1 patch 1 patch		Specify Site Specify Site Right Arm
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch 1 patch, transdermal, Administer over 12 Hours, Every 24 hours, First dose on Tue 1/23/24 at 0015, Apply to R ankle for 12 hours,remove for 12 hours `Patch may be cut. Remove old patch	Medication Applied Medication Applied Medication Applied	02/10/2024 5:47 PM PST 02/09/2024 4:21 PM PST 02/08/2024 3:53 PM PST	1 patch 1 patch 1 patch		Behind Left Ear Specify Site Back
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch 1 patch, transdermal, Administer over 12 Hours, Every 24 hours, First dose (after last modification) on Tue 2/13/24 at 1945, Apply to R hip for 12 hours,remove for 12 hours `Patch may be cut. Remove old patch	Medication Applied Medication Applied Medication Applied	02/17/2024 8:24 AM PST 02/16/2024 9:24 AM PST 02/15/2024 8:25 AM PST	1 patch 1 patch 1 patch		Back Specify Site Back
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 1 patch 1 patch, transdermal, Administer over 12 Hours, Once, On Wed 2/14/24 at 1015, For 1 dose, Apply to affected area for 12 hours,remove for 12 hours `Patch may be cut. Remove old patch	Medication Applied	02/14/2024 10:10 AM PST	1 patch		Back
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch 2 patch, transdermal, Administer over 12 Hours, Every 24 hours, First dose (after last modification) on Sun 2/11/24 at 1900, Apply to left and right neck for 12 hours,remove for 12 hours `Patch may be cut. Remove old patch	Medication Applied Medication Applied Medication Applied	02/17/2024 8:24 AM PST 02/16/2024 9:23 AM PST 02/15/2024 8:25 AM PST	2 patches 2 patches 2 patches		Back Back Back
lidocaine-menthol (LIDOPATCH) 3.6-1.25 % patch 2 patch 2 patch, transdermal, Administer over 12 Hours, Every 24 hours, First dose on Wed 2/14/24 at 1300, Apply to back for 12 hours,remove for 12 hours `Patch may be cut. Remove old patch	Medication Applied Medication Applied	02/16/2024 11:54 AM PST 02/14/2024 2:06 PM PST	2 patches 2 patches		Back Back
lisinopriL (PRINIVIL) tablet 10 mg 10 mg, oral, Daily, First dose on Tue 1/30/24 at 0900, Hold for SBP less than 100. `	Given	01/30/2024 8:29 AM PST	10 mg		
LORazepam (ATIVAN) 2 mg/mL injection 1 mg 1 mg, intraVENOUS, Once, On Sat 1/20/24 at 0030, For 1 dose, Dilute with equal volume of NS or D5W. Administer over 2-5 min. `	Given	01/20/2024 12:11 AM PST	1 mg		
losartan (COZAAR) tablet 12.5 mg 12.5 mg, oral, At bedtime, First dose (after last modification) on Wed 1/31/24 at 2100, `	Given	02/01/2024 9:08 PM PST	12.5 mg		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
losartan (COZAAR) tablet 25 mg 25 mg, oral, At bedtime, First dose (after last modification) on Fri 2/2/24 at 2100, `	Given Given Given	02/04/2024 8:07 PM PST 02/03/2024 10:15 PM PST 02/02/2024 9:05 PM PST	25 mg 25 mg 25 mg		
magnesium sulfate 1 g in 100 mL D5W IVPB Premix 1 g, intraVENOUS, at 100 mL/hr, Administer over 60 Minutes, Every 1 hour scheduled, First dose on Fri 1/12/24 at 2015, For 2 doses, Over 20 minutes `Premix	New Bag New Bag	01/12/2024 10:00 PM PST 01/12/2024 8:05 PM PST	1 g 1 g	100 mL/hr 100 mL/hr	
melatonin tablet 10 mg 10 mg, oral, At bedtime PRN, sleep / insomnia, Starting on Mon 1/15/24 at 0251, `	Given Given Given	02/08/2024 3:39 AM PST 02/03/2024 2:00 AM PST 02/01/2024 9:08 PM PST	10 mg 10 mg 10 mg		
metoCLOPramide (REGLAN) injection 10 mg 10 mg, intraVENOUS, Every 6 hours PRN, nausea or vomiting, not relieved by Ondansetron (ZOFRAN), Starting on Fri 1/12/24 at 2252, Step 2 of 3 - Administer antiemetic agents in sequence numbered. If nausea persists after 30 minutes, proceed to the next numbered medicine (Step 3). `					
metoprolol (LOPRESSOR) 5 mg/5 mL injection 5 mg 5 mg, intraVENOUS, Once, On Mon 1/15/24 at 2345, For 1 dose, Telemetry monitoring required. Individual doses of 10 mg or less may be given as IV push over 1-2 minutes. `	Given	01/15/2024 11:26 PM PST	5 mg		
metroNIDAZOLE (FLAGYL) tablet 500 mg 500 mg, oral, Every 12 hours scheduled, First dose on Fri 1/26/24 at 2100, For 7 days, `, Indications: pulmonary infection	Given Given Given	01/30/2024 8:29 AM PST 01/29/2024 9:02 PM PST 01/29/2024 8:47 AM PST	500 mg 500 mg 500 mg		
metroNIDAZOLE (FLAGYL) tablet 500 mg 500 mg, oral, Every 12 hours scheduled, First dose (after last modification) on Tue 1/30/24 at 2100, For 19 doses, `, Indications: pulmonary infection	Given Given Given	02/08/2024 9:50 PM PST 02/08/2024 10:09 AM PST 02/07/2024 9:50 PM PST	500 mg 500 mg 500 mg		
metroNIDAZOLE in NaCl (iso-os) (FLAGYL) IVPB 500 mg 500 mg, intraVENOUS, at 100 mL/hr, Administer over 60 Minutes, Every 12 hours, First dose on Sat 1/13/24 at 0330, For 7 days, `Do NOT Refrigerate. Premix. Protect from light., Indications: pulmonary infection	New Bag New Bag New Bag	01/19/2024 2:24 PM PST 01/19/2024 3:23 AM PST 01/18/2024 2:24 PM PST	500 mg 500 mg 500 mg	100 mL/hr 100 mL/hr 100 mL/hr	
metroNIDAZOLE in NaCl (iso-os) (FLAGYL) IVPB 500 mg	New Bag New Bag New Bag	01/22/2024 10:04 AM PST 01/21/2024 9:42 PM PST 01/21/2024 9:25 AM PST	500 mg 500 mg 500 mg	100 mL/hr 100 mL/hr 100 mL/hr	

Medication Order	MAR Action	Action Date	Dose	Rate	Site
500 mg, intraVENOUS, at 100 mL/hr, Administer over 60 Minutes, Every 12 hours, First dose (after last reorder) on Sat 1/20/24 at 1030, For 7 days, `Do NOT Refrigerate. Premix. Protect from light., Indications: pulmonary infection					
midazolam (VERSED) injection 2 mg 2 mg, intraVENOUS, Once, On Sat 1/13/24 at 0545, For 1 dose, `	Given	01/13/2024 5:45 AM PST	2 mg		
midazolam (VERSED) injection 5 mg 5 mg, intraVENOUS, Once, On Sat 1/13/24 at 0715, For 1 dose, `	Given	01/13/2024 6:56 AM PST	5 mg		
multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet 1 tablet, oral, Daily, First dose on Mon 1/15/24 at 1400, `	Given Given Given	02/17/2024 8:24 AM PST 02/16/2024 9:19 AM PST 02/15/2024 8:25 AM PST	1 tablet 1 tablet 1 tablet		
naproxen (NAPROSYN) tablet 500 mg 500 mg, oral, 2 times daily PRN, mild pain (level 1-3), Starting on Tue 1/16/24 at 1727, Take with food or snack to lessen stomach upset. `	Given	01/18/2024 2:21 PM PST	500 mg		
naproxen (NAPROSYN) tablet 500 mg 500 mg, oral, 2 times daily PRN, mild pain (level 1-3), moderate pain (level 4-6), headaches, Starting on Fri 1/19/24 at 1400, Take with food or snack to lessen stomach upset. `	Given Given Given	02/03/2024 6:53 AM PST 02/02/2024 6:13 PM PST 02/02/2024 12:53 AM PST	500 mg 500 mg 500 mg		
ondansetron (PF) (ZOFRAN) injection 4 mg 4 mg, intraVENOUS, Every 6 hours PRN, nausea or vomiting, Starting on Fri 1/12/24 at 2252, Step 1 of 3 - Administer antiemetic agents in sequence numbered. If nausea persists after 30 minutes, proceed to the next numbered medicine (Step 2). `	Given Given Given	02/06/2024 2:59 PM PST 02/02/2024 9:06 PM PST 02/02/2024 1:59 AM PST	4 mg 4 mg 4 mg		
oxyCODONE (ROXICODONE) immediate release tablet 5 mg 5 mg, oral, Every 4 hours PRN, moderate pain (level 4-6), Starting on Sat 1/13/24 at 0053, `	Given	01/13/2024 1:05 AM PST	5 mg		
oxyCODONE (ROXICODONE) immediate release tablet 5 mg 5 mg, oral, Every 4 hours PRN, moderate pain (level 4-6), Starting on Wed 1/17/24 at 1147, `	Given Given Given	02/06/2024 8:58 PM PST 02/05/2024 4:29 AM PST 02/03/2024 10:14 PM PST	5 mg 5 mg 5 mg		
oxyCODONE-acetaminophen (PERCOCET) 10-325 mg per tablet 1 tablet 1 tablet, oral, Every 4 hours PRN, severe pain (level 7-10), Starting on Fri 2/16/24 at 1146, Do not exceed 4 g Acetaminophen per day `	Given	02/17/2024 3:48 PM PST	1 tablet		
oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet 1 tablet 1 tablet, oral, Every 4 hours PRN, moderate pain (level 4-6), severe pain (level 7-10), Starting on Sat 2/10/24 at 1806, Do not exceed 4 g Acetaminophen per day `	Given Given	02/16/2024 9:40 AM PST 02/15/2024 11:20 PM PST	1 tablet 1 tablet		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
	Given	02/15/2024 6:01 PM PST	1 tablet		
oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet 1 tablet 1 tablet, oral, Every 4 hours PRN, moderate pain (level 4-6), Starting on Fri 2/16/24 at 1147, Do not exceed 4 g Acetaminophen per day `	Given Given Given	02/17/2024 6:21 AM PST 02/17/2024 2:20 AM PST 02/16/2024 9:13 PM PST	1 tablet 1 tablet 1 tablet		
pantoprazole (PROTONIX) injection 40 mg 40 mg, intraVENOUS, Daily, First dose on Sat 1/13/24 at 0915, `For IVP - reconstitute each 40 mg with 10 mL NS. Administer over 2 minutes via dedicated line or Y-site. Flush before and after dose.	Given Given Given	01/15/2024 8:09 AM PST 01/14/2024 8:35 AM PST 01/13/2024 10:47 AM PST	40 mg 40 mg 40 mg		
PHENYLephrineHCl in 0.9% NaCl (NEO-SYNEPHRINE) 1 mg/10 mL (100 mcg/mL) injection - ADS Override Pull Starting on Sat 1/13/24 at 0542, For 1 dose, Created by cabinet override `Do NOT Refrigerate. Protect from light					
PHENYLephrineHCl in 0.9% NaCl (NEO-SYNEPHRINE) 1 mg/10 mL (100 mcg/mL) injection 1 mg 1 mg, intraVENOUS, Once, On Sat 1/13/24 at 0545, For 1 dose, `Do NOT Refrigerate. Protect from light	Given	01/13/2024 6:06 AM PST	1 mg		
prochlorperazine (COMPAZINE) injection 5 mg 5 mg, intraVENOUS, Every 6 hours PRN, nausea or vomiting, Starting on Fri 1/12/24 at 2252, Max Rate: 5 mg/min. `					
propofoL (DIPRIVAN) 10 mg/mL infusion 0-100 mcg/kg/min × 63.6 kg (0-38.16 mL/hr, rounded to 0-38.2 mL/hr), intraVENOUS, Titrated, Starting on Sat 1/13/24 at 0545, Start at 20 mcg/kg/min. Titrate no faster than 20 mcg/kg/min every 5 min to goal until ordered Target Arousal level achieved. Notify MD if SBP less than 90 mmHg or HR less than 50 BPM. Shake well before use. `Do NOT Refrigerate. Do not administer through the same I.V. catheter with blood or plasma. Tubing and any unused portions of propofol vials should be discarded after 12 hours., Titration Indication: Sedation	Rate/Dose Change New Bag Rate/Dose Verify	01/14/2024 6:05 AM PST 01/14/2024 6:04 AM PST 01/14/2024 2:00 AM PST	40 mcg/kg/min 50 mcg/kg/min 50 mcg/kg/min	15.3 mL/hr 19.1 mL/hr 19.1 mL/hr	
rocuronium (ZEMURON) injection 60 mg 60 mg, intraVENOUS, Once, On Sat 1/13/24 at 0545, For 1 dose, `WARNING - PARALYZING AGENT	Given	01/13/2024 5:46 AM PST	60 mg		
sacubitriL-valsartan (ENTRESTO) 24-26 mg per tablet 1 tablet 1 tablet, oral, 2 times daily, First dose on Sun 2/4/24 at 2145, hold for SBP under 100 `	Given Given Given	02/17/2024 8:23 AM PST 02/16/2024 9:04 PM PST 02/16/2024 9:21 AM PST	1 tablet 1 tablet 1 tablet		
sennosides-docusate sodium (SENOKOT-S) 8.6-50 mg per tablet 1 tablet	Given	02/17/2024 8:23 AM PST	1 tablet		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
1 tablet, oral, 2 times daily, First dose on Fri 1/12/24 at 2315, Hold for excessive water loss in stools, stools with no form, or stooling in excess of three bowel movements per day. `	Given Given	02/16/2024 9:04 PM PST 02/16/2024 9:20 AM PST	1 tablet 1 tablet		
sodium chloride (NS) 0.9 % flush for radiology 50 mL 50 mL, intraVENOUS, Once, On Mon 1/29/24 at 1530, For 1 dose, `	Given	01/29/2024 3:11 PM PST	50 mL		
sodium chloride 0.9 % (NS) infusion 50 mL/hr, intraVENOUS, Continuous, Starting on Wed 1/17/24 at 1445, For 1 day, `Premix	Rate/Dose Verify Rate/Dose Verify Rate/Dose Verify	01/18/2024 12:00 PM PST 01/18/2024 8:00 AM PST 01/18/2024 4:48 AM PST		50 mL/hr 50 mL/hr 50 mL/hr	
sodium chloride 0.9 % bolus 1,000 mL 1,000 mL, intraVENOUS, at 999 mL/hr, Administer over 1 Hours, Once, On Sun 1/21/24 at 0915, For 1 dose, `Premix	New Bag	01/21/2024 9:21 AM PST	1,000 mL	999 mL/hr	
sodium chloride 0.9 % bolus 250 mL 250 mL, intraVENOUS, at 125 mL/hr, Administer over 2 Hours, Once, On Wed 1/17/24 at 0845, For 1 dose, `Premix	New Bag	01/17/2024 9:53 AM PST	250 mL	125 mL/hr	
vancomycin (VANCOCIN) 1 g in sodium chloride 0.9 % (NS) 250 mL Vial2Bag 1 g, intraVENOUS, at 180 mL/hr, Administer over 90 Minutes, Once, On Sat 1/13/24 at 1645, For 1 dose, `Vial-2-Bag medication. Using V2B adapter and diluent bag, assemble and mix before use. For medication administration, scan the vial barcode, scan the pump, then scan the diluent bag. Ensure you have selected the correct diluent bag size for this order., Indications: pulmonary infection	New Bag	01/13/2024 6:28 PM PST	1 g	180 mL/hr	
vancomycin (VANCOCIN) 1 g in sodium chloride 0.9 % (NS) 250 mL Vial2Bag 1 g, intraVENOUS, at 180 mL/hr, Administer over 90 Minutes, Every 24 hours, First dose on Sun 1/14/24 at 1600, `Vial-2-Bag medication. Using V2B adapter and diluent bag, assemble and mix before use. For medication administration, scan the vial barcode, scan the pump, then scan the diluent bag. Ensure you have selected the correct diluent bag size for this order., Indications: pulmonary infection	New Bag	01/14/2024 4:30 PM PST	1 g	180 mL/hr	
vancomycin (VANCOCIN) 750 mg in sodium chloride 0.9 % (NS) 250 mL Vial2Bag 750 mg, intraVENOUS, at 180 mL/hr, Administer over 90 Minutes, Every 12 hours, First dose (after last modification) on Mon 1/15/24 at 1000, `Vial-2-Bag medication. Using V2B adapter and diluent bag, assemble and mix before use. For medication administration, scan the vial barcode, scan the pump, then scan the diluent bag. Ensure you have selected the correct diluent bag size for this order., Indications: pulmonary infection	New Bag New Bag New Bag	01/16/2024 10:38 AM PST 01/15/2024 11:28 PM PST 01/15/2024 10:00 AM PST	750 mg 750 mg 750 mg	180 mL/hr 180 mL/hr 180 mL/hr	
vancomycin (VANCOCIN) 750 mg in sodium chloride 0.9 % (NS) 250 mL Vial2Bag	New Bag	01/26/2024 9:25 AM PST	750 mg	180 mL/hr	

Medication Order	MAR Action	Action Date	Dose	Rate	Site
750 mg, intraVENOUS, at 180 mL/hr, Administer over 90 Minutes, Every 8 hours, First dose on Fri 1/26/24 at 0800, `Vial-2-Bag medication. Using V2B adapter and diluent bag, assemble and mix before use. For medication administration, scan the vial barcode, scan the pump, then scan the diluent bag. Ensure you have selected the correct diluent bag size for this order., Indications: Discitis/ Osteomylitis					
vancomycin (VANCOCIN) IVPB 1,250 mg in sodium chloride 0.9 % (NS) 250 mL (Premix) 1,250 mg (rounded from 1,302 mg = 20 mg/kg × 65.1 kg), intraVENOUS, at 125 mL/hr, Administer over 120 Minutes, Once, On Sat 1/13/24 at 0200, For 1 dose, `Refrigerate. Premix, Indications: pulmonary infection	New Bag	01/13/2024 2:49 AM PST	1,250 mg	125 mL/hr	
vancomycin (VANCOCIN) IVPB 1,250 mg in sodium chloride 0.9 % (NS) 250 mL (Premix) 1,250 mg (rounded from 1,360 mg = 20 mg/kg × 68 kg), intraVENOUS, at 125 mL/hr, Administer over 120 Minutes, Once, On Thu 1/25/24 at 2300, For 1 dose, `Refrigerate. Premix, Indications: Discitis/ Osteomylitis	New Bag	01/25/2024 11:00 PM PST	1,250 mg	125 mL/hr	
Vancomycin Pharmacy Consult (NOT A MEDICATION ORDER) Routine	Acknowledged Acknowledged Acknowledged	01/14/2024 9:00 PM PST 01/14/2024 9:00 AM PST 01/13/2024 8:50 PM PST			
zolpidem (AMBIEN) tablet 5 mg 5 mg, oral, At bedtime PRN, sleep / insomnia, Starting on Mon 1/15/24 at 1230, `	Given Given Given	02/16/2024 11:04 PM PST 02/16/2024 2:27 AM PST 02/14/2024 10:50 PM PST	5 mg 5 mg 5 mg		

Insurance - documented as of this encounter

Guarantor: Petruccio, Elizabeth Tina

Account Type	Relation to Patient	Date of Birth	Phone	Billing Address
Personal/Family	Self	07/29/1964		HOMELESS SAN DIEGO, CA 99999

MEDI-CAL

Member	Subscriber	Plan / Payer (Effective 01/01/2024-03/15/2024)
Name: Petruccio, Elizabeth Tina Member ID: 93931022D Relation to Subscriber: Self	Name: Petruccio, Elizabeth Tina Subscriber ID: 93931022D	Payer ID: Not on file Group ID: Not on file Type: Medi-Cal Phone: 916-636-1200 Address: PO BOX 15700 SACRAMENTO, CA 95852-1700

Advance Directives

For more information, please contact: 858-927-7132 (6AM - 4PM America/Los_Angeles, Monday-Friday)

Full Code (Latest Code Status on File)

Date Activated	Date Inactivated	Comments
1/12/2024 9:06 PM	2/17/2024 6:01 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:20 PM	12/23/2021 8:29 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:09 PM	12/22/2021 10:20 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers

Document Coverage Dates

Jan. 12, 2024 - Feb. 17, 2024

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

Encounter Providers

Encounter Date

Jan. 12, 2024 - Feb. 17, 2024

Stephen Zanoni, MD (Attending)
NPI: 1750585535
619-691-7290 (Work)
619-691-7435 (Fax)
435 H St
Chula Vista, CA 91910-4307
Emergency Medicine

Christopher Jones, MD (Attending, Admitting)
NPI: 1700291390
619-862-6673 (Work)
619-691-7335 (Fax)
435 H Street
CV 31
Chula Vista, CA 91910
Pulmonary Disease

Pedro Dammert Coronado, MD (Attending)
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619-862-6673 (Work)
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435 H St
CV 31
Chula Vista, CA 91910-4307
Pulmonary Disease

Truc Ho, MD (Attending)
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619-691-7335 (Fax)
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Hospitalist

Martha Strauser, MD (Attending)
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619-691-7335 (Fax)
435 H St
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Hospitalist

Diep Dao, DO (Attending)
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619-862-6673 (Work)
619-691-7335 (Fax)
435 H St
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Chula Vista, CA 91910
Hospitalist

Omar Khayyat, MD (Attending)
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619-691-7335 (Fax)
435 H St
CV 31
Chula Vista, CA 91910-4307
Hospitalist

Amanda de Four, MD (Attending)
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619-862-6673 (Work)
619-691-7335 (Fax)
435 H St
CV 31
Chula Vista, CA 91910-4307
Hospitalist

Elizabeth Tina Petruccio

Summary of Care, generated on Oct. 13, 2026

Patient Demographics - Female; born Jul. 29, 1964

Patient Address	Patient Name	Communication
HOMELESS *(Home)* SAN DIEGO, CA 99999 *Former (Nov. 18, 2024 - Dec. 23, 2024):* 759 TH AVE *(Home)* SAN DIEGO, CA 92101 *Former (Dec. 22, 2021 - Nov. 17, 2024):* HOMELESS *(Home)* SAN DIEGO, CA 99999	Elizabeth Tina Petruccio *Former / Aliases:* Elizabeth Petruccio	619-337-5922 (Mobile) Etp949609@gmail.com etp12024@gmail.com etp949609@gmail.com

Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Single

Note from Scripps Health

This document contains information that was shared with Elizabeth Tina Petruccio. It may not contain the entire record from Scripps Health.

Reason for Visit

Reason
Shortness of Breath

Inpatient Admission

Specialty	Diagnoses / Procedures	Referred By Contact	Referred To Contact
	Diagnoses Chest pain, unspecified type Heart failure		

Referral ID	Status	Reason	Start Date	Expiration Date	Visits Requested	Visits Authorized
7423951					1	1

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
12/22/2021 7:41 PM PST - 12/23/2021 6:19 PM PST	Hospital Encounter	Scripps Mercy Hospital Chula Vista 4th FL Definitive Observation Unit 435 H Street Chula Vista, CA 91910-4307 619-691-7000	**Douglas Ballesteros, MD** 435 H St Chula Vista, CA 91910-4307 619-691-7290 (Work) 619-691-7435 (Fax) **Mark Lazarus, MD** 10010 Campus Point Dr San Diego, CA 92121 858-678-6550 (Work) 858-678-6571 (Fax) **John-Vincent Arcilla, MD** 435 H St CV 31 Chula Vista, CA 91910-4307 619-862-6673 (Work) 619-691-7335 (Fax)	Discharge Disposition: Disch to Home (Routine)

Allergies - documented as of this encounter (statuses as of 10/13/2026)

Not on file

Medications - documented as of this encounter (statuses as of 10/13/2026)

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
lisinopriL (PRINIVIL) 10 mg tablet	Take 1 tablet (10 mg total) by mouth daily.	30 tablet			12/24/2021	12/23/2021	Discontinued
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day.	60 tablet			12/23/2021	12/23/2021	Discontinued
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily.	30 tablet			12/24/2021	12/23/2021	Discontinued
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			12/23/2021	12/23/2021	Discontinued
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily for 14 days.	14 tablet			12/23/2021	12/23/2021	Discontinued (Reorder)
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing.	18 g	3		12/23/2021	12/23/2021	Discontinued (Reorder)
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days.	21 capsule			12/23/2021	12/23/2021	Discontinued
lisinopriL (PRINIVIL) 10 mg tablet	Take 1 tablet (10 mg total) by mouth daily.	30 tablet			12/24/2021	12/23/2021	Discontinued
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days.	21 capsule			12/23/2021	12/23/2021	Discontinued
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing.	18 g	3		12/23/2021	12/23/2021	Discontinued (Reorder)
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily for 14 days.	14 tablet			12/23/2021	12/23/2021	Discontinued (Reorder)
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			12/23/2021	12/23/2021	Discontinued
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily.	30 tablet			12/24/2021	12/23/2021	Discontinued
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day.	60 tablet			12/23/2021	12/23/2021	Discontinued
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day. Caloptima Mcal, ID 93931022D30310	60 tablet			12/23/2021	09/18/2024	Discontinued
lisinopriL (PRINIVIL) 10 mg tablet	Take 1 tablet (10 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	02/17/2024	Discontinued (Stop Taking at Discharge)
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days. Caloptima Mcal ID 93931022D30310	21 capsule			12/23/2021	01/15/2024	Discontinued (DC EXPIRED MED)
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g	3		12/23/2021	09/18/2024	Discontinued (Reorder)
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily for 14 days. Caloptima Mcal ID 93931022D30310	14 tablet			12/23/2021	01/15/2024	Discontinued (Med Hx - Alternate Therapy)

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date	Status
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/23/2021	01/15/2024	Discontinued (Historical Discontinue/ Completed)
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	09/18/2024	Discontinued

Active Problems - documented as of this encounter (statuses as of 10/13/2026)

Problem	Noted Date	Diagnosed Date
Fluid in the lungs	12/23/2021	
Asthma	12/23/2021	
High blood pressure	12/23/2021	
Heart failure	12/22/2021	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Former				
Smokeless Tobacco: Never				

Pregnant	Comments
No	

Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	06/22/2025 10:19 PM PDT
Legal Sex	Female	12/22/2021 7:22 PM PST
Gender Identity	Female	06/22/2025 10:19 PM PDT
Sexual Orientation	Not on file	

COVID-19 Exposure	Response	Date Recorded
In the last month, have you been in contact with someone who was confirmed or suspected to have Coronavirus / COVID-19?	No / Unsure	12/22/2021 8:10 PM PST

Last Filed Vital Signs - documented in this encounter

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	132/82	12/23/2021 3:31 PM PST	
Pulse	108	12/23/2021 3:31 PM PST	
Temperature	36.6 °C (97.9 °F)	12/23/2021 3:31 PM PST	
Respiratory Rate	20	12/23/2021 3:31 PM PST	
Oxygen Saturation	93%	12/23/2021 3:31 PM PST	
Inhaled Oxygen Concentration	-	-	
Weight	69.2 kg (152 lb 8.9 oz)	12/23/2021 4:01 AM PST	
Height	170.2 cm (5' 7")	12/22/2021 11:22 PM PST	
Body Mass Index	23.89	12/22/2021 11:22 PM PST	

Functional Status - documented as of this encounter

Glasgow Coma Scale

Question	Answer	Date of Assessment	Author
Eye Opening	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Best Verbal Response	5	12/23/2021 7:50 AM PST	Your Scripps Health Team
Best Motor Response	6	12/23/2021 7:50 AM PST	Your Scripps Health Team
Glasgow Coma Scale Score	15	12/23/2021 7:50 AM PST	Your Scripps Health Team

Prehospital Treatment

Question	Answer	Date of Assessment	Author
Additional PTA Meds	DUONEB	12/22/2021 8:06 PM PST	Nurse Amanda C
Transport Service	MEDIC 32	12/22/2021 8:06 PM PST	Nurse Amanda C
Patient Category	Yellow/ Moderate	12/22/2021 8:06 PM PST	Nurse Amanda C
From Hospital/Clinic/Care Facility?	No	12/22/2021 8:06 PM PST	Nurse Amanda C
Run #	21212106	12/22/2021 8:06 PM PST	Nurse Amanda C
Prehospital Treatment	Yes	12/22/2021 8:06 PM PST	Nurse Amanda C

Pre Hospital Vitals

Question	Answer	Date of Assessment	Author
Temp	97.5	12/22/2021 8:06 PM PST	Nurse Amanda C
Temp src	Tympanic	12/22/2021 8:06 PM PST	Nurse Amanda C
Pulse	120	12/22/2021 8:06 PM PST	Nurse Amanda C
Resp	24	12/22/2021 8:06 PM PST	Nurse Amanda C
BP	148/112	12/22/2021 8:06 PM PST	Nurse Amanda C
SpO2	98	12/22/2021 8:06 PM PST	Nurse Amanda C

Is there a suspected infection?

Answer	Date of Assessment	Author
No	12/22/2021 8:07 PM PST	Nurse Amanda C

Is the patient's mental status altered?

Answer	Date of Assessment	Author
No	12/22/2021 8:07 PM PST	Nurse Amanda C

Columbia Suicide Severity Rating Scale

Question	Answer	Date of Assessment	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
1. Have you wished you were dead or wished you could go to sleep and not wake up? (RETIRED/READ-ONLY)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
2. Have you actually had any thoughts of killing yourself? (RETIRED, READ-ONLY)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Vitals

Question	Answer	Date of Assessment	Author
Weight	2440.93	12/23/2021 4:01 AM PST	Your Scripps Health Team
Weight Method	Bed Scale	12/23/2021 4:01 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
MAP (Monitor)	99	12/23/2021 3:31 PM PST	Your Scripps Health Team

Height and Weight

Question	Answer	Date of Assessment	Author
BSA (Calculated - sq m)	1.8	12/22/2021 11:22 PM PST	Your Scripps Health Team

Respiratory

Question	Answer	Date of Assessment	Author
Bilateral Breath Sounds	Clear;Diminished	12/23/2021 7:50 AM PST	Your Scripps Health Team
Dyspnea Occurrence	With exertion	12/23/2021 7:50 AM PST	Your Scripps Health Team
Chest Assessment	Chest expansion symmetrical;Trachea midline	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory Pattern	Normal	12/23/2021 7:50 AM PST	Your Scripps Health Team
Cough	Non-productive	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory Effort	Unlabored	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory Depth/Rhythm	Regular	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Oxygen Therapy

Question	Answer	Date of Assessment	Author
Oxygen Therapy	None (Room air)	12/23/2021 3:31 PM PST	Your Scripps Health Team
Patient Activity	At rest	12/23/2021 3:31 PM PST	Your Scripps Health Team
Pulse Oximetry Type	Continuous	12/22/2021 10:00 PM PST	Nurse Amanda C

Skin Color/Condition

Question	Answer	Date of Assessment	Author
Skin Color/Condition (WDL)	WDL	12/22/2021 8:12 PM PST	Nurse Amanda C

Endocrine

Question	Answer	Date of Assessment	Author
Endocrine (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	12/22/2021 8:12 PM PST	Nurse Amanda C

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	12/22/2021 8:12 PM PST	Nurse Amanda C

Quick Updates

Question	Answer	Date of Assessment	Author
Quick Updates - Free Text	Box#15, called Tele tech Ashley	12/22/2021 10:47 PM PST	Your Scripps Health Team

Violence Risk Screening (Step 1)

Question	Answer	Date of Assessment	Author
Automatic Triggers	NA	12/22/2021 11:25 PM PST	Your Scripps Health Team
Risk Factor Triggers	Substance Abuse	12/22/2021 11:25 PM PST	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Date of Assessment	Author
Violence Screening Outcome	Negative Violence Risk Screen	12/22/2021 11:25 PM PST	Your Scripps Health Team

Precaution Necessity Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Chlorhexidine Daily Bath Results	No CHG bathing required	12/22/2021 11:25 PM PST	Your Scripps Health Team
Precautions	Follow Standard Precautions	12/22/2021 11:25 PM PST	Your Scripps Health Team
Patient Status	None of the above	12/22/2021 11:25 PM PST	Your Scripps Health Team
Known Infection Status	None	12/22/2021 11:25 PM PST	Your Scripps Health Team
Patient has an Open Draining Wound that cannot be contained by dressing	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

C.difficile Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	No Active Diarrhea	12/22/2021 11:25 PM PST	Your Scripps Health Team
Risks Present	None of the Above	12/22/2021 11:25 PM PST	Your Scripps Health Team
Results	No C. difficile test necessary	12/22/2021 11:25 PM PST	Your Scripps Health Team

CRO and Candida auris Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
(RETIRED) Overnight Hospital/Clinic stay and/or Invasive procedure outside U.S. in past 12 months	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
CRO Results and Interventions	Specimen Not Required.	12/22/2021 11:25 PM PST	Your Scripps Health Team
History of LTAC/SNF/Subacute residency within the last 12 months	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
History of overnight hospitalization and/or invasive procedure outside of the United States within last 12 months	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
(RETIRED) History of colonization/infection with CRO (carbapenem-resistant organisms)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
(RETIRED) Patient has a tracheostomy and/or is mechanically ventilated at outside facility	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Candida auris Results and Interventions	Specimen Not Required.	12/22/2021 11:25 PM PST	Your Scripps Health Team

Assistive Devices at Bedside

Question	Answer	Date of Assessment	Author
Patient Assistive Devices at Bedside	None	12/23/2021 7:50 AM PST	Your Scripps Health Team

Assistive Devices Sent Home

Question	Answer	Date of Assessment	Author
Patient Assistive Devices Sent Home	None	12/22/2021 11:25 PM PST	Your Scripps Health Team

Patient Medications

Question	Answer	Date of Assessment	Author
Medications brought by patient?	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Valuables Sent to Safe/Secure Area

Question	Answer	Date of Assessment	Author
Patient Valuables Sent to Safe/Secure Area	No Valuables	12/22/2021 11:25 PM PST	Your Scripps Health Team

Output (mL)

Question	Answer	Date of Assessment	Author
Urine	200	12/23/2021 3:31 PM PST	Your Scripps Health Team
Urinary Incontinence	No	12/23/2021 3:31 PM PST	Your Scripps Health Team
Urine Color	Yellow/ straw	12/23/2021 3:31 PM PST	Your Scripps Health Team
Urine Appearance	Clear	12/23/2021 3:31 PM PST	Your Scripps Health Team

Sedation Scales

Question	Answer	Date of Assessment	Author
Pasero Opioid-Induced Sedation Scale (POSS)	1	12/23/2021 3:08 PM PST	Your Scripps Health Team
Richmond Agitation Sedation Scale (RASS)	RASS 0	12/23/2021 3:08 PM PST	Your Scripps Health Team

Feature 3: Altered Level of Consciousness

Answer	Date of Assessment	Author
Negative	12/23/2021 3:08 PM PST	Your Scripps Health Team

Braden Scale

Question	Answer	Date of Assessment	Author
Sensory Perceptions	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Moisture	4	12/23/2021 7:50 AM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Activity	3	12/23/2021 7:50 AM PST	Your Scripps Health Team
Mobility	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Nutrition	3	12/23/2021 7:50 AM PST	Your Scripps Health Team
Friction and Shear	3	12/23/2021 7:50 AM PST	Your Scripps Health Team
Braden Scale Score	21	12/23/2021 7:50 AM PST	Your Scripps Health Team

History of Falling

Answer	Date of Assessment	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Secondary Diagnosis

Answer	Date of Assessment	Author
15	12/23/2021 7:50 AM PST	Your Scripps Health Team

Ambulatory Aids

Answer	Date of Assessment	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Intravenous Therapy/Heparin/Saline Lock

Answer	Date of Assessment	Author
20	12/23/2021 7:50 AM PST	Your Scripps Health Team

Gait/Transferring

Answer	Date of Assessment	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Mental Status

Answer	Date of Assessment	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Morse Fall Risk Score

Answer	Date of Assessment	Author
35	12/23/2021 7:50 AM PST	Your Scripps Health Team

ARS (Complete on admission, prior to resuming PO intake, and post procedure)

Question	Answer	Date of Assessment	Author
Is the Patient Intubated? (If yes, defer ARS for 4 hours after extubation)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

ARS-Tier 1

Question	Answer	Date of Assessment	Author
Is the alertness level INsufficient to remain awake for 10 minutes while sitting upright?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Is voice weak, wet or abnormal in any way? (Yes if patient cannot speak)	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Does the patient drool?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Is speech slurred?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Is the patients cough weak or inaudible?, if no cough then YES	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Aspiration Risk Result	Pass, complete Oral Intake Status	12/22/2021 11:25 PM PST	Your Scripps Health Team

ARS-Contraindications for Water Test

Question	Answer	Date of Assessment	Author
Is the patient Strict NPO? (If yes, defer ARS until diet order placed)	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Admitted for Bariatric Surgery?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Are there current speech therapy orders for swallow evaluation of dysphagia?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Has the patient/caregiver reported a history of difficulty swallowing?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team

ARS-Water Test Required - only if all the questions are 'no'

Question	Answer	Date of Assessment	Author
Water Test Required?	Yes (See Below)	12/22/2021 11:25 PM PST	Your Scripps Health Team

Meds to Beds for Discharge Prescriptions

Question	Answer	Date of Assessment	Author
Will the patient be utilizing the meds to beds program?	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Medication Resource Planning

Question	Answer	Date of Assessment	Author
How will patient obtain medications for discharge?	I need resources on how to obtain my discharge medications	12/22/2021 11:25 PM PST	Your Scripps Health Team

Assistance Levels - How much assistance does the patient need?

Question	Answer	Date of Assessment	Author
To advance to sitting edge of bed and to maintain sitting balance?	None - Independent	12/22/2021 11:25 PM PST	Your Scripps Health Team
To safely advance to standing (with Gait belt and Assistive device as indicated)?	None - Independent	12/22/2021 11:25 PM PST	Your Scripps Health Team
To march in place or to transfer to bedside chair/commode (with gait belt and Assistive Device as Indicated)?	None - Independent	12/22/2021 11:25 PM PST	Your Scripps Health Team
To safely ambulate to bathroom or doorway (with gait belt and Assistive Device as indicated)?	None - Independent	12/22/2021 11:25 PM PST	Your Scripps Health Team
Does the patient report that they are mobilizing at their functional baseline, without concerns for safe home mobility? (Retired)	Yes	12/22/2021 11:25 PM PST	Your Scripps Health Team

Advance Directives (For Healthcare)

Question	Answer	Date of Assessment	Author
Have you reviewed your Advance Directive and is it current?	Yes	12/22/2021 11:25 PM PST	Your Scripps Health Team
Advance Directive	Patient does not have advance directive	12/22/2021 11:25 PM PST	Your Scripps Health Team
Information Provided on Healthcare Directives	Yes	12/22/2021 11:25 PM PST	Your Scripps Health Team
Healthcare Agent Appointed	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Pre-existing DNR/DNI Order	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Patient Requests Assistance	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

ADL Functional Risk Screen

Question	Answer	Date of Assessment	Author
OT - Recent Onset	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
ST - Recent Onset	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Results (OT and SLP)	No Risks Identified	12/22/2021 11:25 PM PST	Your Scripps Health Team

Nurse Mobility Screen

Question	Answer	Date of Assessment	Author
Exclusion Critera	No Exclusion Criteria	12/22/2021 11:25 PM PST	Your Scripps Health Team

Mobility Screening Outcome

Question	Answer	Date of Assessment	Author
Mobility Screening Outcome(Retired)	Patient is independent/ baseline. Nursing will follow activity orders. No PT/OT indicated.	12/22/2021 11:25 PM PST	Your Scripps Health Team

Discharge Planning

Question	Answer	Date of Assessment	Author
RETIRED Has discharge transport been arranged?	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
(Retired) Assistance Needed	None	12/22/2021 11:25 PM PST	Your Scripps Health Team
Type of Residence	Private residence	12/22/2021 11:25 PM PST	Your Scripps Health Team
RETIRED Does the patient need discharge transport arranged?	Yes	12/22/2021 11:25 PM PST	Your Scripps Health Team
(Retired) Discharge	Home	12/22/2021 11:25 PM PST	Your Scripps Health Team

Initial Discharge Planning

Question	Answer	Date of Assessment	Author
Designated Family Caregiver	Documented in Emergency Contacts	12/23/2021 11:47 AM PST	Your Scripps Health Team
Support Systems	Church/faith community;Children	12/23/2021 11:47 AM PST	Your Scripps Health Team
Home Care Services	No	12/23/2021 11:47 AM PST	Your Scripps Health Team
(Retired) Patient expects to be discharged to:	Pt is working w/ her church group to get transportation back to Orange County and shelter. Alternatively, church members may provide transportation and shelter in Mexico before going to Orange County.	12/23/2021 11:47 AM PST	Your Scripps Health Team
(Retired) Assistive Devices	None	12/23/2021 11:47 AM PST	Your Scripps Health Team

Living Arrangements

Question	Answer	Date of Assessment	Author
Living Arrangements	Other	12/23/2021 11:56 AM PST	Your Scripps Health Team

Screen for MRSA (all items must be valued)

Question	Answer	Date of Assessment	Author
Admission to ICU	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Hospitalized within last 30 days	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Inpatient dialysis treatment	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Admit/Transfer from a Skilled Nursing Facility (SNF)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Known history of MRSA	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Results and Interventions	Specimen Not Required.	12/22/2021 11:25 PM PST	Your Scripps Health Team

Tuberculosis Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	Denies symptoms	12/22/2021 11:25 PM PST	Your Scripps Health Team
Risks Present	Denies risks	12/22/2021 11:25 PM PST	Your Scripps Health Team
Results	TB Risk Screen Negative	12/22/2021 11:25 PM PST	Your Scripps Health Team

Malnutrition Screening Tool

Question	Answer	Date of Assessment	Author
Have you recently lost weight without trying?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Weight Loss Score	0	12/22/2021 11:25 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
If yes, how much weight have you lost?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Have you been eating poorly because of your decreased appetite?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Appetite Score	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Malnutrition Screening Outcome	0	12/22/2021 11:25 PM PST	Your Scripps Health Team

Trauma/Abuse Screening

Question	Answer	Date of Assessment	Author
Physical Abuse	Denies	12/22/2021 11:25 PM PST	Your Scripps Health Team
Verbal Abuse	Denies	12/22/2021 11:25 PM PST	Your Scripps Health Team
Sexual Abuse	Denies	12/22/2021 11:25 PM PST	Your Scripps Health Team

Consults

Question	Answer	Date of Assessment	Author
Spiritual Care Consult Needed	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Social Services Consult Needed	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Sleep Apnea Screening

Question	Answer	Date of Assessment	Author
Do you SNORE loudly?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Do you often feel tired, fatigued, or sleepy during the daytime?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Has anyone observed you stop breathing during your sleep?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Do you have or are you being treated for high blood pressure?	1	12/22/2021 11:25 PM PST	Your Scripps Health Team
Is patient obese/ very overweight – BMI more than 35 kg/m2?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Age over 50 years old?	1	12/22/2021 11:25 PM PST	Your Scripps Health Team
Neck Circumference >16 inches?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Is patient male?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Score	2	12/22/2021 11:25 PM PST	Your Scripps Health Team

Values/Beliefs

Question	Answer	Date of Assessment	Author
Cultural Practices During Hospitalization	English speaking, Caucasian woman	12/23/2021 11:56 AM PST	Your Scripps Health Team
Spiritual Practices During Hospitalization	Catholic	12/23/2021 11:56 AM PST	Your Scripps Health Team

Precautions

Question	Answer	Date of Assessment	Author
Precautions	Fall risk	12/23/2021 8:00 AM PST	Your Scripps Health Team

Safe Environment

Question	Answer	Date of Assessment	Author
ID Bands and Medical Alerts	ID;Fall	12/23/2021 8:00 AM PST	Your Scripps Health Team
Instructed on How to Call for Help	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Overbed Table Within Reach	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Bed In Lowest Position	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Bed Wheels Locked	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Side Rails/Bed Safety	3/4	12/23/2021 8:00 AM PST	Your Scripps Health Team
NonSkid Footwear	On;Patient in bed	12/23/2021 8:00 AM PST	Your Scripps Health Team
Safety Equipment at Bedside	Suction	12/23/2021 8:00 AM PST	Your Scripps Health Team

Fall Risk Interventions

Question	Answer	Date of Assessment	Author
Room Door Open	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Alarm On	Bed	12/23/2021 8:00 AM PST	Your Scripps Health Team
Fall signage in place	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Wristband fall alert on	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Gait Belt Used	Not applicable	12/23/2021 8:00 AM PST	Your Scripps Health Team
Other fall interventions	call light in reach, rounding	12/23/2021 8:00 AM PST	Your Scripps Health Team

Mobility

Question	Answer	Date of Assessment	Author
Level of Assistance	One person assist	12/23/2021 4:00 PM PST	Your Scripps Health Team
Mode of Transport	Bed	12/23/2021 12:00 AM PST	Your Scripps Health Team
Mobilization Response	Tolerated well	12/23/2021 4:00 AM PST	Your Scripps Health Team
Repositioned	Turns self	12/23/2021 4:00 PM PST	Your Scripps Health Team
Head of Bed Elevated	HOB 30	12/23/2021 4:00 PM PST	Your Scripps Health Team
Heels/Feet	Heels elevated off bed	12/23/2021 12:00 PM PST	Your Scripps Health Team
Range of Motion - Active	All extremities	12/23/2021 4:00 PM PST	Your Scripps Health Team

Hygiene

Question	Answer	Date of Assessment	Author
Level of Assistance	Minimal assist	12/23/2021 2:00 PM PST	Your Scripps Health Team
Hygiene	Peri care/ Incontinent care	12/23/2021 2:00 PM PST	Your Scripps Health Team
Chlorhexidine Bath	Yes	12/23/2021 12:00 AM PST	Your Scripps Health Team

Comfort and Environment Interventions

Question	Answer	Date of Assessment	Author
Comfort	Repositioned	12/23/2021 4:00 PM PST	Your Scripps Health Team
Restraint Alternatives	Bed alarms;Provide sleep/rest;Low bed	12/23/2021 4:00 PM PST	Your Scripps Health Team

Breastfeeding Status

Answer	Date of Assessment	Author
No	12/23/2021 1:10 AM PST	Your Scripps Health Team

Pain Assessment

Question	Answer	Date of Assessment	Author
Pain Orientation	Mid	12/23/2021 10:46 AM PST	Your Scripps Health Team
Pain Interventions	Medication (See MAR);Cold applied	12/23/2021 2:08 PM PST	Your Scripps Health Team
Pain Radiating Towards	neck	12/23/2021 10:46 AM PST	Your Scripps Health Team
Pain Descriptors	Aching;Pressure;Sharp	12/23/2021 2:08 PM PST	Your Scripps Health Team
Pain Onset	Awakened from sleep	12/23/2021 2:08 PM PST	Your Scripps Health Team
Pain Frequency	Constant/continuous	12/23/2021 2:08 PM PST	Your Scripps Health Team
Patient's Stated Pain Goal	Mild - 1-3	12/23/2021 3:08 PM PST	Your Scripps Health Team
Response to Interventions	pain improving	12/23/2021 3:08 PM PST	Your Scripps Health Team
Pain Aggravating Factors	Reposition/ movement	12/23/2021 5:47 AM PST	Your Scripps Health Team
Effect of Pain on Daily Activities	Activity;Life Enjoyment;Sleep	12/23/2021 5:47 AM PST	Your Scripps Health Team
Pain Type	Acute pain	12/23/2021 10:46 AM PST	Your Scripps Health Team
Clinical Progression of Pain	Not changed	12/23/2021 5:47 AM PST	Your Scripps Health Team
VDS Pain Score	Mild Pain (Pain Level 1-3)	12/23/2021 3:08 PM PST	Your Scripps Health Team
Pain Assessment	0-10	12/23/2021 3:08 PM PST	Your Scripps Health Team
Pain Location	Head	12/23/2021 10:46 AM PST	Your Scripps Health Team
Pain Score	3	12/23/2021 3:08 PM PST	Your Scripps Health Team

Oncology

Question	Answer	Date of Assessment	Author
Does patient have an active cancer diagnosis?	No	12/23/2021 7:50 AM PST	Your Scripps Health Team

Neurological

Question	Answer	Date of Assessment	Author
Neuro (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

HEENT

Question	Answer	Date of Assessment	Author
HEENT (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Pacemaker

Question	Answer	Date of Assessment	Author
Pacemaker	No	12/23/2021 7:50 AM PST	Your Scripps Health Team

Cardiac

Question	Answer	Date of Assessment	Author
Heart Sounds	S1, S2	12/23/2021 7:50 AM PST	Your Scripps Health Team
Jugular Venous Distention (JVD)	No	12/23/2021 7:50 AM PST	Your Scripps Health Team
Cardiac (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Date of Assessment	Author
Skin Color	Appropriate for ethnicity	12/23/2021 7:50 AM PST	Your Scripps Health Team
Skin Condition/Temp	Warm;Dry	12/23/2021 7:50 AM PST	Your Scripps Health Team

Skin

Question	Answer	Date of Assessment	Author
Skin Integrity	Bruising	12/23/2021 7:50 AM PST	Your Scripps Health Team
Skin Location	bilateral hip	12/23/2021 7:50 AM PST	Your Scripps Health Team
Skin Turgor	Non-tenting	12/23/2021 7:50 AM PST	Your Scripps Health Team
Medication Patch Assessments	Nitroglycerin	12/22/2021 11:30 PM PST	Your Scripps Health Team
Skin (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Medication Patch Assessment

Question	Answer	Date of Assessment	Author
NITROGLYCERIN Medication Patch Location	Chest	12/22/2021 11:30 PM PST	Your Scripps Health Team

Peripheral Vascular

Question	Answer	Date of Assessment	Author
Peripheral Vascular (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Gastrointestinal

Question	Answer	Date of Assessment	Author
Gastrointestinal (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Genitourinary

Question	Answer	Date of Assessment	Author
Genitourinary (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Anus/Rectum

Question	Answer	Date of Assessment	Author
Anus/Rectum Observations	WDL	12/22/2021 11:30 PM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Charting Type

Question	Answer	Date of Assessment	Author
Charting Type	Shift assessment	12/23/2021 7:50 AM PST	Your Scripps Health Team

Intake

Question	Answer	Date of Assessment	Author
P.O.	240	12/23/2021 5:33 PM PST	Your Scripps Health Team
Percent Meals Eaten (%)	100	12/23/2021 5:33 PM PST	Your Scripps Health Team
Snacks	turkey sandwich and pudding	12/22/2021 11:30 PM PST	Your Scripps Health Team
Meal	Dinner	12/23/2021 5:33 PM PST	Your Scripps Health Team

Unmeasured Output

Question	Answer	Date of Assessment	Author
Unmeasured Urine Occurrence	1	12/23/2021 2:54 PM PST	Your Scripps Health Team

Stool Output/Assessment

Question	Answer	Date of Assessment	Author
Unmeasured Stool Occurrence	0	12/23/2021 6:00 AM PST	Your Scripps Health Team

Nutrition

Question	Answer	Date of Assessment	Author
Feeding	Able to feed self	12/23/2021 8:00 AM PST	Your Scripps Health Team

Glasgow Coma Scale

Question	Answer	Date of Assessment	Author
Eye Opening	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Best Verbal Response	5	12/23/2021 7:50 AM PST	Your Scripps Health Team
Best Motor Response	6	12/23/2021 7:50 AM PST	Your Scripps Health Team
Glasgow Coma Scale Score	15	12/23/2021 7:50 AM PST	Your Scripps Health Team

Prehospital Treatment

Question	Answer	Date of Assessment	Author
Additional PTA Meds	DUONEB	12/22/2021 8:06 PM PST	Nurse Amanda C
Transport Service	MEDIC 32	12/22/2021 8:06 PM PST	Nurse Amanda C
Patient Category	Yellow/ Moderate	12/22/2021 8:06 PM PST	Nurse Amanda C
From Hospital/Clinic/Care Facility?	No	12/22/2021 8:06 PM PST	Nurse Amanda C
Run #	21212106	12/22/2021 8:06 PM PST	Nurse Amanda C
Prehospital Treatment	Yes	12/22/2021 8:06 PM PST	Nurse Amanda C

Pre Hospital Vitals

Question	Answer	Date of Assessment	Author
Temp	97.5	12/22/2021 8:06 PM PST	Nurse Amanda C
Temp src	Tympanic	12/22/2021 8:06 PM PST	Nurse Amanda C
Pulse	120	12/22/2021 8:06 PM PST	Nurse Amanda C
Resp	24	12/22/2021 8:06 PM PST	Nurse Amanda C
BP	148/112	12/22/2021 8:06 PM PST	Nurse Amanda C
SpO2	98	12/22/2021 8:06 PM PST	Nurse Amanda C

Is there a suspected infection?

Answer	Date of Assessment	Author
No	12/22/2021 8:07 PM PST	Nurse Amanda C

Is the patient's mental status altered?

Answer	Date of Assessment	Author
No	12/22/2021 8:07 PM PST	Nurse Amanda C

Columbia Suicide Severity Rating Scale

Question	Answer	Date of Assessment	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
1. Have you wished you were dead or wished you could go to sleep and not wake up? (RETIRED/READ-ONLY)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
2. Have you actually had any thoughts of killing yourself? (RETIRED, READ-ONLY)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Respiratory

Question	Answer	Date of Assessment	Author
Bilateral Breath Sounds	Clear;Diminished	12/23/2021 7:50 AM PST	Your Scripps Health Team
Dyspnea Occurrence	With exertion	12/23/2021 7:50 AM PST	Your Scripps Health Team
Chest Assessment	Chest expansion symmetrical;Trachea midline	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory Pattern	Normal	12/23/2021 7:50 AM PST	Your Scripps Health Team
Cough	Non-productive	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory Effort	Unlabored	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory Depth/Rhythm	Regular	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Oxygen Therapy

Question	Answer	Date of Assessment	Author
Oxygen Therapy	None (Room air)	12/23/2021 3:31 PM PST	Your Scripps Health Team
Patient Activity	At rest	12/23/2021 3:31 PM PST	Your Scripps Health Team
Pulse Oximetry Type	Continuous	12/22/2021 10:00 PM PST	Nurse Amanda C

HEENT

Question	Answer	Date of Assessment	Author
HEENT (WDL)	WDL	12/22/2021 8:12 PM PST	Nurse Amanda C

Endocrine

Question	Answer	Date of Assessment	Author
Endocrine (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	12/22/2021 8:12 PM PST	Nurse Amanda C

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	12/22/2021 8:12 PM PST	Nurse Amanda C

Vitals

Question	Answer	Date of Assessment	Author
MAP (Monitor)	99	12/23/2021 3:31 PM PST	Your Scripps Health Team

Violence Risk Screening (Step 1)

Question	Answer	Date of Assessment	Author
Automatic Triggers	NA	12/22/2021 11:25 PM PST	Your Scripps Health Team
Risk Factor Triggers	Substance Abuse	12/22/2021 11:25 PM PST	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Date of Assessment	Author
Violence Screening Outcome	Negative Violence Risk Screen	12/22/2021 11:25 PM PST	Your Scripps Health Team

Precaution Necessity Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Chlorhexidine Daily Bath Results	No CHG bathing required	12/22/2021 11:25 PM PST	Your Scripps Health Team
Precautions	Follow Standard Precautions	12/22/2021 11:25 PM PST	Your Scripps Health Team
Patient Status	None of the above	12/22/2021 11:25 PM PST	Your Scripps Health Team
Known Infection Status	None	12/22/2021 11:25 PM PST	Your Scripps Health Team
Patient has an Open Draining Wound that cannot be contained by dressing	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

C.difficile Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	No Active Diarrhea	12/22/2021 11:25 PM PST	Your Scripps Health Team
Risks Present	None of the Above	12/22/2021 11:25 PM PST	Your Scripps Health Team
Results	No C. difficile test necessary	12/22/2021 11:25 PM PST	Your Scripps Health Team

CRO and Candida auris Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
(RETIRED) Overnight Hospital/Clinic stay and/or Invasive procedure outside U.S. in past 12 months	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
CRO Results and Interventions	Specimen Not Required.	12/22/2021 11:25 PM PST	Your Scripps Health Team
History of LTAC/SNF/Subacute residency within the last 12 months	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
History of overnight hospitalization and/or invasive procedure outside of the United States within last 12 months	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
(RETIRED) History of colonization/infection with CRO (carbapenem-resistant organisms)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
(RETIRED) Patient has a tracheostomy and/or is mechanically ventilated at outside facility	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Candida auris Results and Interventions	Specimen Not Required.	12/22/2021 11:25 PM PST	Your Scripps Health Team

Assistive Devices at Bedside

Question	Answer	Date of Assessment	Author
Patient Assistive Devices at Bedside	None	12/23/2021 7:50 AM PST	Your Scripps Health Team

Assistive Devices Sent Home

Question	Answer	Date of Assessment	Author
Patient Assistive Devices Sent Home	None	12/22/2021 11:25 PM PST	Your Scripps Health Team

Patient Medications

Question	Answer	Date of Assessment	Author
Medications brought by patient?	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Valuables Sent to Safe/Secure Area

Question	Answer	Date of Assessment	Author
Patient Valuables Sent to Safe/Secure Area	No Valuables	12/22/2021 11:25 PM PST	Your Scripps Health Team

Sedation Scales

Question	Answer	Date of Assessment	Author
Pasero Opioid-Induced Sedation Scale (POSS)	1	12/23/2021 3:08 PM PST	Your Scripps Health Team
Richmond Agitation Sedation Scale (RASS)	RASS 0	12/23/2021 3:08 PM PST	Your Scripps Health Team

Feature 3: Altered Level of Consciousness

Answer	Date of Assessment	Author
Negative	12/23/2021 3:08 PM PST	Your Scripps Health Team

Braden Scale

Question	Answer	Date of Assessment	Author
Sensory Perceptions	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Moisture	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Activity	3	12/23/2021 7:50 AM PST	Your Scripps Health Team
Mobility	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Nutrition	3	12/23/2021 7:50 AM PST	Your Scripps Health Team
Friction and Shear	3	12/23/2021 7:50 AM PST	Your Scripps Health Team
Braden Scale Score	21	12/23/2021 7:50 AM PST	Your Scripps Health Team

History of Falling

Answer	Date of Assessment	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Secondary Diagnosis

Answer	Date of Assessment	Author
15	12/23/2021 7:50 AM PST	Your Scripps Health Team

Ambulatory Aids

Answer	Date of Assessment	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Intravenous Therapy/Heparin/Saline Lock

Answer	Date of Assessment	Author
20	12/23/2021 7:50 AM PST	Your Scripps Health Team

Gait/Transferring

Answer	Date of Assessment	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Mental Status

Answer	Date of Assessment	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Morse Fall Risk Score

Answer	Date of Assessment	Author
35	12/23/2021 7:50 AM PST	Your Scripps Health Team

ARS (Complete on admission, prior to resuming PO intake, and post procedure)

Question	Answer	Date of Assessment	Author
Is the Patient Intubated? (If yes, defer ARS for 4 hours after extubation)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

ARS-Tier 1

Question	Answer	Date of Assessment	Author
Is the alertness level INsufficient to remain awake for 10 minutes while sitting upright?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Is voice weak, wet or abnormal in any way? (Yes if patient cannot speak)	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Does the patient drool?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Is speech slurred?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Is the patients cough weak or inaudible?, if no cough then YES	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Aspiration Risk Result	Pass, complete Oral Intake Status	12/22/2021 11:25 PM PST	Your Scripps Health Team

ARS-Contraindications for Water Test

Question	Answer	Date of Assessment	Author
Is the patient Strict NPO? (If yes, defer ARS until diet order placed)	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Admitted for Bariatric Surgery?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Are there current speech therapy orders for swallow evaluation of dysphagia?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Has the patient/caregiver reported a history of difficulty swallowing?	0	12/22/2021 11:25 PM PST	Your Scripps Health Team

ARS-Water Test Required - only if all the questions are 'no'

Question	Answer	Date of Assessment	Author
Water Test Required?	Yes (See Below)	12/22/2021 11:25 PM PST	Your Scripps Health Team

Meds to Beds for Discharge Prescriptions

Question	Answer	Date of Assessment	Author
Will the patient be utilizing the meds to beds program?	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Medication Resource Planning

Question	Answer	Date of Assessment	Author
How will patient obtain medications for discharge?	I need resources on how to obtain my discharge medications	12/22/2021 11:25 PM PST	Your Scripps Health Team

Assistance Levels - How much assistance does the patient need?

Question	Answer	Date of Assessment	Author
To advance to sitting edge of bed and to maintain sitting balance?	None - Independent	12/22/2021 11:25 PM PST	Your Scripps Health Team
To safely advance to standing (with Gait belt and Assistive device as indicated)?	None - Independent	12/22/2021 11:25 PM PST	Your Scripps Health Team
To march in place or to transfer to bedside chair/commode (with gait belt and Assistive Device as Indicated)?	None - Independent	12/22/2021 11:25 PM PST	Your Scripps Health Team
To safely ambulate to bathroom or doorway (with gait belt and Assistive Device as indicated)?	None - Independent	12/22/2021 11:25 PM PST	Your Scripps Health Team

Advance Directives (For Healthcare)

Question	Answer	Date of Assessment	Author
Have you reviewed your Advance Directive and is it current?	Yes	12/22/2021 11:25 PM PST	Your Scripps Health Team
Advance Directive	Patient does not have advance directive	12/22/2021 11:25 PM PST	Your Scripps Health Team
Information Provided on Healthcare Directives	Yes	12/22/2021 11:25 PM PST	Your Scripps Health Team
Healthcare Agent Appointed	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Pre-existing DNR/DNI Order	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Patient Requests Assistance	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

ADL Functional Risk Screen

Question	Answer	Date of Assessment	Author
OT - Recent Onset	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
ST - Recent Onset	0	12/22/2021 11:25 PM PST	Your Scripps Health Team
Results (OT and SLP)	No Risks Identified	12/22/2021 11:25 PM PST	Your Scripps Health Team

Nurse Mobility Screen

Question	Answer	Date of Assessment	Author
Exclusion Critera	No Exclusion Criteria	12/22/2021 11:25 PM PST	Your Scripps Health Team

Mobility Screening Outcome

Question	Answer	Date of Assessment	Author
Mobility Screening Outcome(Retired)	Patient is independent/ baseline. Nursing will follow activity orders. No PT/OT indicated.	12/22/2021 11:25 PM PST	Your Scripps Health Team

Discharge Planning

Question	Answer	Date of Assessment	Author
RETIRED Has discharge transport been arranged?	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
(Retired) Assistance Needed	None	12/22/2021 11:25 PM PST	Your Scripps Health Team
Type of Residence	Private residence	12/22/2021 11:25 PM PST	Your Scripps Health Team
RETIRED Does the patient need discharge transport arranged?	Yes	12/22/2021 11:25 PM PST	Your Scripps Health Team
(Retired) Discharge	Home	12/22/2021 11:25 PM PST	Your Scripps Health Team

Initial Discharge Planning

Question	Answer	Date of Assessment	Author
Designated Family Caregiver	Documented in Emergency Contacts	12/23/2021 11:47 AM PST	Your Scripps Health Team
Support Systems	Church/faith community;Children	12/23/2021 11:47 AM PST	Your Scripps Health Team
Home Care Services	No	12/23/2021 11:47 AM PST	Your Scripps Health Team
(Retired) Patient expects to be discharged to:	Pt is working w/ her church group to get transportation back to Orange County and shelter. Alternatively, church members may provide transportation and shelter in Mexico before going to Orange County.	12/23/2021 11:47 AM PST	Your Scripps Health Team
(Retired) Assistive Devices	None	12/23/2021 11:47 AM PST	Your Scripps Health Team

Living Arrangements

Question	Answer	Date of Assessment	Author
Living Arrangements	Other	12/23/2021 11:56 AM PST	Your Scripps Health Team

Screen for MRSA (all items must be valued)

Question	Answer	Date of Assessment	Author
Admission to ICU	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Hospitalized within last 30 days	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Inpatient dialysis treatment	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Admit/Transfer from a Skilled Nursing Facility (SNF)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Known history of MRSA	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Results and Interventions	Specimen Not Required.	12/22/2021 11:25 PM PST	Your Scripps Health Team

Tuberculosis Screening (all items must be valued)

Question	Answer	Date of Assessment	Author
Signs/Symptoms Present	Denies symptoms	12/22/2021 11:25 PM PST	Your Scripps Health Team
Risks Present	Denies risks	12/22/2021 11:25 PM PST	Your Scripps Health Team
Results	TB Risk Screen Negative	12/22/2021 11:25 PM PST	Your Scripps Health Team

Trauma/Abuse Screening

Question	Answer	Date of Assessment	Author
Physical Abuse	Denies	12/22/2021 11:25 PM PST	Your Scripps Health Team
Verbal Abuse	Denies	12/22/2021 11:25 PM PST	Your Scripps Health Team
Sexual Abuse	Denies	12/22/2021 11:25 PM PST	Your Scripps Health Team

Consults

Question	Answer	Date of Assessment	Author
Spiritual Care Consult Needed	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Social Services Consult Needed	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Values/Beliefs

Question	Answer	Date of Assessment	Author
Cultural Practices During Hospitalization	English speaking, Caucasian woman	12/23/2021 11:56 AM PST	Your Scripps Health Team
Spiritual Practices During Hospitalization	Catholic	12/23/2021 11:56 AM PST	Your Scripps Health Team

Precautions

Question	Answer	Date of Assessment	Author
Precautions	Fall risk	12/23/2021 8:00 AM PST	Your Scripps Health Team

Safe Environment

Question	Answer	Date of Assessment	Author
ID Bands and Medical Alerts	ID;Fall	12/23/2021 8:00 AM PST	Your Scripps Health Team
Instructed on How to Call for Help	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Overbed Table Within Reach	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Bed In Lowest Position	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Bed Wheels Locked	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Side Rails/Bed Safety	3/4	12/23/2021 8:00 AM PST	Your Scripps Health Team
NonSkid Footwear	On;Patient in bed	12/23/2021 8:00 AM PST	Your Scripps Health Team
Safety Equipment at Bedside	Suction	12/23/2021 8:00 AM PST	Your Scripps Health Team

Fall Risk Interventions

Question	Answer	Date of Assessment	Author
Room Door Open	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Alarm On	Bed	12/23/2021 8:00 AM PST	Your Scripps Health Team
Fall signage in place	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Wristband fall alert on	Yes	12/23/2021 8:00 AM PST	Your Scripps Health Team
Gait Belt Used	Not applicable	12/23/2021 8:00 AM PST	Your Scripps Health Team
Other fall interventions	call light in reach, rounding	12/23/2021 8:00 AM PST	Your Scripps Health Team

Mobility

Question	Answer	Date of Assessment	Author
Level of Assistance	One person assist	12/23/2021 4:00 PM PST	Your Scripps Health Team
Mode of Transport	Bed	12/23/2021 12:00 AM PST	Your Scripps Health Team
Mobilization Response	Tolerated well	12/23/2021 4:00 AM PST	Your Scripps Health Team
Repositioned	Turns self	12/23/2021 4:00 PM PST	Your Scripps Health Team
Head of Bed Elevated	HOB 30	12/23/2021 4:00 PM PST	Your Scripps Health Team
Heels/Feet	Heels elevated off bed	12/23/2021 12:00 PM PST	Your Scripps Health Team
Range of Motion - Active	All extremities	12/23/2021 4:00 PM PST	Your Scripps Health Team

Hygiene

Question	Answer	Date of Assessment	Author
Level of Assistance	Minimal assist	12/23/2021 2:00 PM PST	Your Scripps Health Team

Question	Answer	Date of Assessment	Author
Hygiene	Peri care/ Incontinent care	12/23/2021 2:00 PM PST	Your Scripps Health Team
Chlorhexidine Bath	Yes	12/23/2021 12:00 AM PST	Your Scripps Health Team

Comfort and Environment Interventions

Question	Answer	Date of Assessment	Author
Comfort	Repositioned	12/23/2021 4:00 PM PST	Your Scripps Health Team
Restraint Alternatives	Bed alarms;Provide sleep/rest;Low bed	12/23/2021 4:00 PM PST	Your Scripps Health Team

Pain Assessment

Question	Answer	Date of Assessment	Author
Pain Orientation	Mid	12/23/2021 10:46 AM PST	Your Scripps Health Team
Pain Interventions	Medication (See MAR);Cold applied	12/23/2021 2:08 PM PST	Your Scripps Health Team
Pain Radiating Towards	neck	12/23/2021 10:46 AM PST	Your Scripps Health Team
Pain Descriptors	Aching;Pressure;Sharp	12/23/2021 2:08 PM PST	Your Scripps Health Team
Pain Onset	Awakened from sleep	12/23/2021 2:08 PM PST	Your Scripps Health Team
Pain Frequency	Constant/continuous	12/23/2021 2:08 PM PST	Your Scripps Health Team
Patient's Stated Pain Goal	Mild - 1-3	12/23/2021 3:08 PM PST	Your Scripps Health Team
Response to Interventions	pain improving	12/23/2021 3:08 PM PST	Your Scripps Health Team
Pain Aggravating Factors	Reposition/ movement	12/23/2021 5:47 AM PST	Your Scripps Health Team
Effect of Pain on Daily Activities	Activity;Life Enjoyment;Sleep	12/23/2021 5:47 AM PST	Your Scripps Health Team
Pain Type	Acute pain	12/23/2021 10:46 AM PST	Your Scripps Health Team
Clinical Progression of Pain	Not changed	12/23/2021 5:47 AM PST	Your Scripps Health Team
VDS Pain Score	Mild Pain (Pain Level 1-3)	12/23/2021 3:08 PM PST	Your Scripps Health Team
Pain Assessment	0-10	12/23/2021 3:08 PM PST	Your Scripps Health Team
Pain Location	Head	12/23/2021 10:46 AM PST	Your Scripps Health Team
Pain Score	3	12/23/2021 3:08 PM PST	Your Scripps Health Team

Oncology

Question	Answer	Date of Assessment	Author
Does patient have an active cancer diagnosis?	No	12/23/2021 7:50 AM PST	Your Scripps Health Team

Neurological

Question	Answer	Date of Assessment	Author
Neuro (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

HEENT

Question	Answer	Date of Assessment	Author
Teeth	Missing teeth	12/23/2021 7:50 AM PST	Your Scripps Health Team
HEENT (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Pacemaker

Question	Answer	Date of Assessment	Author
Pacemaker	No	12/23/2021 7:50 AM PST	Your Scripps Health Team

Cardiac

Question	Answer	Date of Assessment	Author
Heart Sounds	S1, S2	12/23/2021 7:50 AM PST	Your Scripps Health Team
Jugular Venous Distention (JVD)	No	12/23/2021 7:50 AM PST	Your Scripps Health Team
Cardiac (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Date of Assessment	Author
Skin Color	Appropriate for ethnicity	12/23/2021 7:50 AM PST	Your Scripps Health Team
Skin Condition/Temp	Warm;Dry	12/23/2021 7:50 AM PST	Your Scripps Health Team

Skin

Question	Answer	Date of Assessment	Author
Skin Integrity	Bruising	12/23/2021 7:50 AM PST	Your Scripps Health Team
Skin Location	bilateral hip	12/23/2021 7:50 AM PST	Your Scripps Health Team
Skin Turgor	Non-tenting	12/23/2021 7:50 AM PST	Your Scripps Health Team
Medication Patch Assessments	Nitroglycerin	12/22/2021 11:30 PM PST	Your Scripps Health Team
Skin (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Medication Patch Assessment

Question	Answer	Date of Assessment	Author
NITROGLYCERIN Medication Patch Location	Chest	12/22/2021 11:30 PM PST	Your Scripps Health Team

Peripheral Vascular

Question	Answer	Date of Assessment	Author
Peripheral Vascular (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Musculoskeletal

Question	Answer	Date of Assessment	Author
Musculoskeletal (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Gastrointestinal

Question	Answer	Date of Assessment	Author
Gastrointestinal (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Genitourinary

Question	Answer	Date of Assessment	Author
Genitourinary (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Anus/Rectum

Question	Answer	Date of Assessment	Author
Anus/Rectum Observations	WDL	12/22/2021 11:30 PM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Date of Assessment	Author
Psychosocial (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Charting Type

Question	Answer	Date of Assessment	Author
Charting Type	Shift assessment	12/23/2021 7:50 AM PST	Your Scripps Health Team

Output (mL)

Question	Answer	Date of Assessment	Author
Urinary Incontinence	No	12/23/2021 3:31 PM PST	Your Scripps Health Team
Urine Color	Yellow/ straw	12/23/2021 3:31 PM PST	Your Scripps Health Team
Urine Appearance	Clear	12/23/2021 3:31 PM PST	Your Scripps Health Team

Nutrition

Question	Answer	Date of Assessment	Author
Feeding	Able to feed self	12/23/2021 8:00 AM PST	Your Scripps Health Team

Mental Status - documented in this encounter

Glasgow Coma Scale

Question	Answer	Entry Date	Author
Eye Opening	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Best Verbal Response	5	12/23/2021 7:50 AM PST	Your Scripps Health Team
Best Motor Response	6	12/23/2021 7:50 AM PST	Your Scripps Health Team
Glasgow Coma Scale Score	15	12/23/2021 7:50 AM PST	Your Scripps Health Team

Prehospital Treatment

Question	Answer	Entry Date	Author
Additional PTA Meds	DUONEB	12/22/2021 8:06 PM PST	Nurse Amanda C
Transport Service	MEDIC 32	12/22/2021 8:06 PM PST	Nurse Amanda C
Patient Category	Yellow/ Moderate	12/22/2021 8:06 PM PST	Nurse Amanda C
From Hospital/Clinic/Care Facility?	No	12/22/2021 8:06 PM PST	Nurse Amanda C
Run #	21212106	12/22/2021 8:06 PM PST	Nurse Amanda C
Prehospital Treatment	Yes	12/22/2021 8:06 PM PST	Nurse Amanda C

Pre Hospital Vitals

Question	Answer	Entry Date	Author
Temp	97.5	12/22/2021 8:06 PM PST	Nurse Amanda C
Temp src	Tympanic	12/22/2021 8:06 PM PST	Nurse Amanda C
Pulse	120	12/22/2021 8:06 PM PST	Nurse Amanda C
Resp	24	12/22/2021 8:06 PM PST	Nurse Amanda C
BP	148/112	12/22/2021 8:06 PM PST	Nurse Amanda C
SpO2	98	12/22/2021 8:06 PM PST	Nurse Amanda C

Is there a suspected infection?

Answer	Entry Date	Author
No	12/22/2021 8:07 PM PST	Nurse Amanda C

Is the patient's mental status altered?

Answer	Entry Date	Author
No	12/22/2021 8:07 PM PST	Nurse Amanda C

Columbia Suicide Severity Rating Scale

Question	Answer	Entry Date	Author
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
1. Have you wished you were dead or wished you could go to sleep and not wake up? (RETIRED/READ-ONLY)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
2. Have you actually had any thoughts of killing yourself? (RETIRED, READ-ONLY)	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Respiratory

Question	Answer	Entry Date	Author
Bilateral Breath Sounds	Clear;Diminished	12/23/2021 7:50 AM PST	Your Scripps Health Team
Dyspnea Occurrence	With exertion	12/23/2021 7:50 AM PST	Your Scripps Health Team

Question	Answer	Entry Date	Author
Chest Assessment	Chest expansion symmetrical;Trachea midline	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory Pattern	Normal	12/23/2021 7:50 AM PST	Your Scripps Health Team
Cough	Non-productive	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory Effort	Unlabored	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory Depth/Rhythm	Regular	12/23/2021 7:50 AM PST	Your Scripps Health Team
Respiratory (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Oxygen Therapy

Question	Answer	Entry Date	Author
Oxygen Therapy	None (Room air)	12/23/2021 3:31 PM PST	Your Scripps Health Team
Patient Activity	At rest	12/23/2021 3:31 PM PST	Your Scripps Health Team
Pulse Oximetry Type	Continuous	12/22/2021 10:00 PM PST	Nurse Amanda C

Endocrine

Question	Answer	Entry Date	Author
Endocrine (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Entry Date	Author
Psychosocial (WDL)	WDL	12/22/2021 8:12 PM PST	Nurse Amanda C

Vitals

Question	Answer	Entry Date	Author
MAP (Monitor)	99	12/23/2021 3:31 PM PST	Your Scripps Health Team

Violence Risk Screening (Step 1)

Question	Answer	Entry Date	Author
Automatic Triggers	NA	12/22/2021 11:25 PM PST	Your Scripps Health Team
Risk Factor Triggers	Substance Abuse	12/22/2021 11:25 PM PST	Your Scripps Health Team

Violence Risk Screening Outcome (Step 2)

Question	Answer	Entry Date	Author
Violence Screening Outcome	Negative Violence Risk Screen	12/22/2021 11:25 PM PST	Your Scripps Health Team

Sedation Scales

Question	Answer	Entry Date	Author
Pasero Opioid-Induced Sedation Scale (POSS)	1	12/23/2021 3:08 PM PST	Your Scripps Health Team

Question	Answer	Entry Date	Author
Richmond Agitation Sedation Scale (RASS)	RASS 0	12/23/2021 3:08 PM PST	Your Scripps Health Team

Feature 3: Altered Level of Consciousness

Answer	Entry Date	Author
Negative	12/23/2021 3:08 PM PST	Your Scripps Health Team

Braden Scale

Question	Answer	Entry Date	Author
Sensory Perceptions	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Moisture	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Activity	3	12/23/2021 7:50 AM PST	Your Scripps Health Team
Mobility	4	12/23/2021 7:50 AM PST	Your Scripps Health Team
Nutrition	3	12/23/2021 7:50 AM PST	Your Scripps Health Team
Friction and Shear	3	12/23/2021 7:50 AM PST	Your Scripps Health Team
Braden Scale Score	21	12/23/2021 7:50 AM PST	Your Scripps Health Team

History of Falling

Answer	Entry Date	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Secondary Diagnosis

Answer	Entry Date	Author
15	12/23/2021 7:50 AM PST	Your Scripps Health Team

Ambulatory Aids

Answer	Entry Date	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Intravenous Therapy/Heparin/Saline Lock

Answer	Entry Date	Author
20	12/23/2021 7:50 AM PST	Your Scripps Health Team

Gait/Transferring

Answer	Entry Date	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Mental Status

Answer	Entry Date	Author
0	12/23/2021 7:50 AM PST	Your Scripps Health Team

Morse Fall Risk Score

Answer	Entry Date	Author
35	12/23/2021 7:50 AM PST	Your Scripps Health Team

Discharge Planning

Question	Answer	Entry Date	Author
RETIRED Has discharge transport been arranged?	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
(Retired) Assistance Needed	None	12/22/2021 11:25 PM PST	Your Scripps Health Team
Type of Residence	Private residence	12/22/2021 11:25 PM PST	Your Scripps Health Team
RETIRED Does the patient need discharge transport arranged?	Yes	12/22/2021 11:25 PM PST	Your Scripps Health Team
(Retired) Discharge	Home	12/22/2021 11:25 PM PST	Your Scripps Health Team

Initial Discharge Planning

Question	Answer	Entry Date	Author
Designated Family Caregiver	Documented in Emergency Contacts	12/23/2021 11:47 AM PST	Your Scripps Health Team
Support Systems	Church/faith community;Children	12/23/2021 11:47 AM PST	Your Scripps Health Team
Home Care Services	No	12/23/2021 11:47 AM PST	Your Scripps Health Team
(Retired) Patient expects to be discharged to:	Pt is working w/ her church group to get transportation back to Orange County and shelter. Alternatively, church members may provide transportation and shelter in Mexico before going to Orange County.	12/23/2021 11:47 AM PST	Your Scripps Health Team

Living Arrangements

Question	Answer	Entry Date	Author
Living Arrangements	Other	12/23/2021 11:56 AM PST	Your Scripps Health Team

Trauma/Abuse Screening

Question	Answer	Entry Date	Author
Physical Abuse	Denies	12/22/2021 11:25 PM PST	Your Scripps Health Team
Verbal Abuse	Denies	12/22/2021 11:25 PM PST	Your Scripps Health Team
Sexual Abuse	Denies	12/22/2021 11:25 PM PST	Your Scripps Health Team

Consults

Question	Answer	Entry Date	Author
Spiritual Care Consult Needed	No	12/22/2021 11:25 PM PST	Your Scripps Health Team
Social Services Consult Needed	No	12/22/2021 11:25 PM PST	Your Scripps Health Team

Values/Beliefs

Question	Answer	Entry Date	Author
Cultural Practices During Hospitalization	English speaking, Caucasian woman	12/23/2021 11:56 AM PST	Your Scripps Health Team
Spiritual Practices During Hospitalization	Catholic	12/23/2021 11:56 AM PST	Your Scripps Health Team

Pain Assessment

Question	Answer	Entry Date	Author
Pain Orientation	Mid	12/23/2021 10:46 AM PST	Your Scripps Health Team
Pain Interventions	Medication (See MAR);Cold applied	12/23/2021 2:08 PM PST	Your Scripps Health Team
Pain Radiating Towards	neck	12/23/2021 10:46 AM PST	Your Scripps Health Team
Pain Descriptors	Aching;Pressure;Sharp	12/23/2021 2:08 PM PST	Your Scripps Health Team
Pain Onset	Awakened from sleep	12/23/2021 2:08 PM PST	Your Scripps Health Team
Pain Frequency	Constant/continuous	12/23/2021 2:08 PM PST	Your Scripps Health Team
Patient's Stated Pain Goal	Mild - 1-3	12/23/2021 3:08 PM PST	Your Scripps Health Team
Response to Interventions	pain improving	12/23/2021 3:08 PM PST	Your Scripps Health Team
Pain Aggravating Factors	Reposition/ movement	12/23/2021 5:47 AM PST	Your Scripps Health Team
Effect of Pain on Daily Activities	Activity;Life Enjoyment;Sleep	12/23/2021 5:47 AM PST	Your Scripps Health Team
Pain Type	Acute pain	12/23/2021 10:46 AM PST	Your Scripps Health Team
Clinical Progression of Pain	Not changed	12/23/2021 5:47 AM PST	Your Scripps Health Team
VDS Pain Score	Mild Pain (Pain Level 1-3)	12/23/2021 3:08 PM PST	Your Scripps Health Team
Pain Assessment	0-10	12/23/2021 3:08 PM PST	Your Scripps Health Team
Pain Location	Head	12/23/2021 10:46 AM PST	Your Scripps Health Team
Pain Score	3	12/23/2021 3:08 PM PST	Your Scripps Health Team

Oncology

Question	Answer	Entry Date	Author
Does patient have an active cancer diagnosis?	No	12/23/2021 7:50 AM PST	Your Scripps Health Team

Neurological

Question	Answer	Entry Date	Author
Neuro (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

HEENT

Question	Answer	Entry Date	Author
HEENT (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Pacemaker

Question	Answer	Entry Date	Author
Pacemaker	No	12/23/2021 7:50 AM PST	Your Scripps Health Team

Cardiac

Question	Answer	Entry Date	Author
Heart Sounds	S1, S2	12/23/2021 7:50 AM PST	Your Scripps Health Team
Jugular Venous Distention (JVD)	No	12/23/2021 7:50 AM PST	Your Scripps Health Team
Cardiac (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Skin Color/Condition

Question	Answer	Entry Date	Author
Skin Color	Appropriate for ethnicity	12/23/2021 7:50 AM PST	Your Scripps Health Team
Skin Condition/Temp	Warm;Dry	12/23/2021 7:50 AM PST	Your Scripps Health Team

Skin

Question	Answer	Entry Date	Author
Skin Integrity	Bruising	12/23/2021 7:50 AM PST	Your Scripps Health Team
Skin Location	bilateral hip	12/23/2021 7:50 AM PST	Your Scripps Health Team
Skin Turgor	Non-tenting	12/23/2021 7:50 AM PST	Your Scripps Health Team
Medication Patch Assessments	Nitroglycerin	12/22/2021 11:30 PM PST	Your Scripps Health Team
Skin (WDL)	X	12/23/2021 7:50 AM PST	Your Scripps Health Team

Medication Patch Assessment

Question	Answer	Entry Date	Author
NITROGLYCERIN Medication Patch Location	Chest	12/22/2021 11:30 PM PST	Your Scripps Health Team

Peripheral Vascular

Question	Answer	Entry Date	Author
Peripheral Vascular (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Musculoskeletal

Question	Answer	Entry Date	Author
Musculoskeletal (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Gastrointestinal

Question	Answer	Entry Date	Author
Gastrointestinal (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Genitourinary

Question	Answer	Entry Date	Author
Genitourinary (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Anus/Rectum

Question	Answer	Entry Date	Author
Anus/Rectum Observations	WDL	12/22/2021 11:30 PM PST	Your Scripps Health Team

Psychosocial

Question	Answer	Entry Date	Author
Psychosocial (WDL)	WDL	12/23/2021 7:50 AM PST	Your Scripps Health Team

Charting Type

Question	Answer	Entry Date	Author
Charting Type	Shift assessment	12/23/2021 7:50 AM PST	Your Scripps Health Team

Output (mL)

Question	Answer	Entry Date	Author
Urinary Incontinence	No	12/23/2021 3:31 PM PST	Your Scripps Health Team
Urine Color	Yellow/ straw	12/23/2021 3:31 PM PST	Your Scripps Health Team
Urine Appearance	Clear	12/23/2021 3:31 PM PST	Your Scripps Health Team

Discharge Summaries - documented in this encounter

John-Vincent Arcilla, MD - 12/23/2021 3:22 PM PST

Formatting of this note might be different from the original.

[image]

Discharge/Transfer Summary

Patient Name: Elizabeth Tina Petruccio **MR#:** 810760324

Attending Provider: John-Vincent Lim Arcilla, MD
Primary Care Physician at Discharge: No primary care provider on file. None
Consultations
IP CONSULT TO CASE MANAGEMENT
IP CONSULT TO SOCIAL WORK

Admission Date: 12/22/2021 **Discharge Date:** 12/23/21

Chief Complaint: shortness of breath and leg swelling

[table#1]:
Discharge Diagnosis

[table#2]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)
•	Pulmonary edema
•	Asthma
•	Hypertension

Unless otherwise stated, all of the above were present on admission

Procedures
* No surgery found *

[table#3]:
Brief HPI
57 y.o. female with history of CHF, hypertension, and asthma presented with progressively worsening shortness of breath and leg swelling for the past 2 weeks. She sleeps with 4 pillows and cannot walk more than a few steps without SOB. She also notes some increased wheezing with the weather changes and needs to use her inhaler daily. Continues to take Lasix 40mg daily and Symbicort inhaler but has not taken Lisinopril and Carvedilol for the past 1 year due to insurance issues. She is not a smoker and has no smoking history. In the ED, BNP 9890 and CXR showing cardiomegaly and left mid lobe opacities suggestive of atelectasis. She was also hypertensive in the ED with BP 154/100. EKG showed sinus tachycardia with LVH. She was started on Lasix 40mg IV BID with strict fluid restriction to 1.2 L/ day. BP improved with Lasix. She was restarted on home Lisinopril.

See admission history and physical exam for detailed information.

[table#4]:
Hospital Course
57 y.o.female with likely nonischemic cardiomyopathy diagnosed 1 year ago with a negative stress, prior methamphetamine abuse and medication nonadherence presents with dyspnea and lower extremity edema found to have an acute on chronic CHF exacerbation.

**# Acute on chronic congestive heart failure exacerbation **
Diagnosed 1 year ago with negative stress test. Has been taking Lasix 40mg daily that she buys from Mexico and uses Symbicort inhaler daily as needed. Nonadherent with Lisinopril or Carvedilol x 1 year due to lack of PCP. CXR showed cardiomegaly and proBNP 9,890 on admission. Patient was started on Lasix 40mg IV with appropriate urine output and improvement of shortness of breath with diuresis. Patient was restarted on home Lisinopril 10mg PO. Patient was given the option of echo to determine the source of CHF but the patient declined and wants to do it outpatient. Patient looks clinically stable for discharge.
- Prescribed Lasix 40mg once per day x 2 weeks.
- Recommend outpatient echo.
- F/u outpatient with PCP and cardiology within 2 weeks.

 # Accelerated hypertensive without obvious endorgan damage, resolved
Patient was hypertensive at 154/100 on admission with improvement after first dose of Lasix, currently 130/87. No signs of other organ damage. Blood pressure was stable during admission and well controlled. Elevated troponin likely demand ischemia.
- Prescribed home medication Coreg 3.125mg PO and resume outpatient
- Prescribed home medication Lisinopril 10mg PO resume outpatient
-F/u outpatient with PCP
–Advised low-salt diet.

#Elevated Troponin likely demand ischemia without signs of ACS
ED course showed elevated Troponin of 0.035 and was given Aspirin and Nitro. Due to lack of chest pain, and ECG changes not consistent with ischemia, no repeat troponin was ordered. Sign and symptoms consistent with acute decompensated heart failure with a known history. Elevated troponin likely due to demand ischemia in the setting of acute exacerbation of chronic heart failure.

 # Asthma
Hx of asthma with use of Symbicort and albuterol in
haler. No acute exacerbation recently. Lung exam without wheezing and only notable for crackles at the bases consistent with CHF with cough likely cardiac in nature.
-The patient has adequate refills of Symbicort but is requesting albuterol inhaler at discharge.
-F/u with PCP

 # History of methamphetamine use
Distant use of snorting methamphetamine. Has not used in many years. Urine toxicology negative on admission.

Physical Exam on DC
BP 121/82 (BP Location: Right arm, Patient Position: Lying) | Pulse 109 | Temp 36.4 °C (97.5 °F) (Oral) | Resp 18 | Ht 1.702 m (5' 7") | Wt 69.2 kg (152 lb 8.9 oz) | LMP (LMP Unknown) | SpO2 93% | Breastfeeding No | BMI 23.89 kg/m^2
**Gen: **WDWN in NAD. Pleasant, cooperative. Speaking in full sentences. No accessory muscle use.
HEENT: NCAT. PERRL. Conjunctivae clear. EOM intact. Partially edentulous
Neck: Normal range of motion. Neck supple.
**CV: **RRR. No gallop, friction rub, or murmur. Distal pulses intact. No JVD.
Lungs: CTA b/l. No respiratory distress. No wheezes. Bibasilar crackles.
Abd: Soft, non-distended. BSx4. There is no rigidity, no guarding, no rebound. Mild epigatric tenderness.
MSK: No deformity or joint swelling.
**Ext: **No clubbing, cyanosis, or edema.
**GU: **No suprapubic tenderness. No CVA tenderness.
Neuro: A&O to person, place, and time. Moves all four extremities spontaneously.

Skin: Skin is warm. No rash noted. No erythema. Non-diaphoretic.
**Psychiatric: **Mood, memory, affect and judgment normal.

**Significant Labs or Imaging **

[table#5]:
Selected Labs (Up to last 2 results from the past 48 hours)	Report
	12/22 2027	12/23 0520
	Sodium	[table#5.1]	[table#5.2]
	Potassium	[table#5.3]	[table#5.4]
	Chloride	[table#5.5]	[table#5.6]
	CO2, Total	[table#5.7]	[table#5.8]
	Urea Nitrogen	[table#5.9]	[table#5.10]
	Creatinine	[table#5.11]	[table#5.12]
	HGB	[table#5.13]	[table#5.14]
	HCT	[table#5.15]	[table#5.16]
	WBC (auto)	[table#5.17]	[table#5.18]
	Platelet Count	[table#5.19]	[table#5.20]

[table#5.1]:
139

[table#5.2]:
138

[table#5.3]:
4.1

[table#5.4]:
4.2

[table#5.5]:
99

[table#5.6]:
97[image:Low]

[table#5.7]:
35[image:High]

[table#5.8]:
36[image:High]

[table#5.9]:
| |

| --- | --- |
| **28[image:High]**
|

[table#5.10]:
31[image:High]

[table#5.11]:
0.7

[table#5.12]:
0.9

[table#5.13]:
12.1

[table#5.14]:
11.9

[table#5.15]:
39.3

[table#5.16]:
38.0

[table#5.17]:
9.2

[table#5.18]:
8.0

[table#5.19]:
348

[table#5.20]:
342

[table#6]:

X-ray portable chest single frontal view

Result Date: 12/22/2021
IMPRESSION: Left midlung and retrocardiac opacities likely reflect atelectasis, although a developing infection cannot be completely excluded. Right lung is clear. No pulmonary edema. Moderate cardiomegaly. Small left pleural effusion. No pneumothorax.

Pending Tests

[table#7]:
Discharge Medications

[table#8]:
Your medication list
START taking these medications
	Instructions	Last Dose Given	Next Dose Due
*
*albuterol HFA** 90 mcg/actuation inhaler
Commonly known as: PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA

| Inhale 2 puffs every 4 (four) hours as needed for wheezing.
| | |
| **aspirin** 81 mg tablet
Start taking on: **December 24, 2021**

| Take 1 tablet (81 mg total) by mouth daily.
| | |
| **benzonatate** 100 mg capsule
Commonly known as: TESSALON

| Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days.
| | |
| **carvediloL** 3.125 mg tablet
Commonly known as: COREG

| Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day.
| | |
| **furosemide** 40 mg tablet
Commonly known as: LASIX

| Take 1 tablet (40 mg total) by mouth daily for 14 days.
| | |
| **lisinopriL** 10 mg tablet
Commonly known as: PRINIVIL
Start taking on: **December 24, 2021**

| Take 1 tablet (10 mg total) by mouth daily.
| | |
| **multivit-iron-FA-calcium-mins** 9 mg iron-400 mcg tablet
Commonly known as: Thera M Plus

| Take 1 tablet by mouth daily.
| | |

[table#9]:
Where to Get Your Medications

| **These medications were sent to Medical Center Pharmacy #2 - Chula Vista, CA - 480 4th Ave** | 480 4th Ave, Chula Vista CA 91910-4410 |
| | Phone: 619-427-1444 |
- albuterol HFA 90 mcg/actuation inhaler
- aspirin 81 mg tablet
- benzonatate 100 mg capsule
- carvediloL 3.125 mg tablet
- furosemide 40 mg tablet
- lisinopriL 10 mg tablet
- multivit-iron-FA-calcium-mins 9 mg iron-400 mcg tablet
[table#10]:

[table#11]:
Follow-up Instructions

[table#12]:
	Follow-up with primary physician (PCP) in 1 week
	When: in 1 week

[table#13]:
Please do the following:
	Follow up with cardiologist
	Specialist: Cardiologist
	When: in 1 week
	Instructions for follow-up: for repeat echocardiogram
	Follow-up with primary physician (PCP) in 1 week
	When: in 1 week
F/u with outpatient PCP and cardiology within 2 weeks.

PT/OT/SLT Recommendations
[table#14]:
PT/OT/SLP Recommendations
No data to display
[table#15]:

Discharge Condition Stable

**Disposition: **Home

Advance Directive/POLST:
Full Code

Soloman Kreik PGY 1 DO

My total discharging and care coordination time was > 30 minutes

**Medical Student: **Erika Hollywood, MS3

Patient discussed in full with attending, Dr. John-Vincent Lim Arcilla, MD

Attending physician attestation:
I have reviewed this case with resident physician including:
1) History and Subjective findings.
2) Physical exam.
3) Laboratory results, X rays and other pertinent studies.

I discussed the assessment and plan with the resident physician.

57 y.o. female w/ Hx chronic heart failure (unknown EF) on furosemide 40 mg PO daily, HTN, and intermittent asthma admitted on 12/22/21 for decompensated heart failure.

She reports medication non-compliance. Her dyspnea on exertion improved s/p IV diuresis (UOP 1450 in 12 hours). She had no chest pain, dyspnea at rest, or ST changes on EKG to suggest ACS.

John Arcilla, M.D.
Internal Medicine

Electronically signed by Soloman Kreik, MD at 12/23/2021 2:58 PM PST
Electronically signed by John-Vincent Arcilla, MD at 12/23/2021 3:22 PM PST

Medications at Time of Discharge - documented as of this encounter

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g	3		12/23/2021	09/18/2024
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	09/18/2024
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days. Caloptima Mcal ID 93931022D30310	21 capsule			12/23/2021	01/15/2024
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day. Caloptima Mcal, ID 93931022D30310	60 tablet			12/23/2021	09/18/2024
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily for 14 days. Caloptima Mcal ID 93931022D30310	14 tablet			12/23/2021	01/15/2024
lisinopriL (PRINIVIL) 10 mg tablet	Take 1 tablet (10 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	02/17/2024
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/23/2021	01/15/2024

Ordered Prescriptions - documented in this encounter

Prescription	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	09/18/2024
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/23/2021	01/15/2024
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily for 14 days. Caloptima Mcal ID 93931022D30310	14 tablet			12/23/2021	01/15/2024
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing. Caloptima Mcal ID 93931022D30310	18 g	3		12/23/2021	09/18/2024
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days. Caloptima Mcal ID 93931022D30310	21 capsule			12/23/2021	01/15/2024
lisinopriL (PRINIVIL) 10 mg tablet	Take 1 tablet (10 mg total) by mouth daily. Caloptima Mcal ID 93931022D30310	30 tablet			12/24/2021	02/17/2024
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day. Caloptima Mcal, ID 93931022D30310	60 tablet			12/23/2021	09/18/2024
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day.	60 tablet			12/23/2021	12/23/2021
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily.	30 tablet			12/24/2021	12/23/2021
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			12/23/2021	12/23/2021
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily for 14 days.	14 tablet			12/23/2021	12/23/2021
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing.	18 g	3		12/23/2021	12/23/2021
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days.	21 capsule			12/23/2021	12/23/2021
lisinopriL (PRINIVIL) 10 mg tablet	Take 1 tablet (10 mg total) by mouth daily.	30 tablet			12/24/2021	12/23/2021
benzonatate (TESSALON) 100 mg capsule	Take 1 capsule (100 mg total) by mouth 3 (three) times a day as needed for cough for up to 7 days.	21 capsule			12/23/2021	12/23/2021
albuterol HFA (PROVENTIL HFA,VENTOLIN HFA,PROAIR HFA) 90 mcg/actuation inhaler	Inhale 2 puffs every 4 (four) hours as needed for wheezing.	18 g	3		12/23/2021	12/23/2021
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg total) by mouth daily for 14 days.	14 tablet			12/23/2021	12/23/2021
multivit-iron-FA-calcium-mins (Thera M Plus) 9 mg iron-400 mcg tablet	Take 1 tablet by mouth daily.	30 tablet			12/23/2021	12/23/2021
aspirin 81 mg tablet	Take 1 tablet (81 mg total) by mouth daily.	30 tablet			12/24/2021	12/23/2021
carvediloL (COREG) 3.125 mg tablet	Take 1 tablet (3.125 mg total) by mouth 2 (two) times a day.	60 tablet			12/23/2021	12/23/2021

Prescription	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
lisinopriL (PRINIVIL) 10 mg tablet	Take 1 tablet (10 mg total) by mouth daily.	30 tablet			12/24/2021	12/23/2021

Progress Notes - documented in this encounter

John-Vincent Arcilla, MD - 12/23/2021 3:21 PM PST

Formatting of this note might be different from the original.

[image]

Hospital Medicine History and Physical

PCP: No primary care provider on file.

[table#1]:
CHIEF COMPLAINT

Shortness of Breath (57-YR-OLD FEMALE WITH HX OF CHF BIBM FROM BOARDER WITH C/O SOB)

[table#2]:
HISTORY OF THE PRESENT ILLNESS

57 y.o.female history of cardiomyopathy notes 1 year ago in the setting of negative stress test, history of prior methamphetamine abuse, asthma who has been off of her medications since she lost her insurance 1 year ago presents with shortness of breath. Patient previously took carvedilol and lisinopril but is not taking those for nearly 1 year. She continues to take Lasix 40 mg daily but she does not have a primary care doctor or cardiologist. She lives in Mexico and gets her medications and that contract.

Over the past 2 weeks she endorses worsening shortness of breath dyspnea on exertion 4 pillow orthopnea and worsening pubic region lower extremity bilateral edema. She suspects that she is in congestive heart failure. Endorses not snorting methamphetamine for over a year. She drinks 1 alcoholic beverage daily but not to the point of intoxication does not have tremors or evidence of withdrawal.

Emergency department course notable for unremarkable BMP, troponin 0 0.035 proBNP 9890 and normal CBC with the exception of an MCV of 101. Influenza and Covid negative. Chest x-ray with cardiomegaly but otherwise unremarkable. KG was sinus tachycardia LVH left atrial large man

[table#3]:
INTERVAL HISTORY
- She denies chest pain and dyspnea at rest. She is ambulating around the room on RA without dyspnea on exertion.

[+] indicates the presence of a symptom, [-] indicates the absence of a symptom
Constitutional: [-] fever
Cardiovascular: [-] chest pain
Gastrointestinal: [-] abdominal pain

[table#4]:
PHYSICAL EXAM

Vital Signs:
Temp: [36.4 °C (97.5 °F)-37.2 °C (98.9 °F)] 36.4 °C (97.5 °F)
Heart Rate: [109-118] 109
Resp: [18-21] 18
BP: (117-154)/(82-113) 121/82
SpO2: 93 %

Physical Exam: All vital signs reviewed and O2 saturation by my interpretation

General- Alert and oriented x4/4 appears older than stated age anxiousnormocephalic atraumatic, pupils equal round reactive sclera
HEENT-partially edentulous, anicteric trachea midline, mucous membranes moist
Lungs-decreased breath sounds with bibasilar rales present
Cardiovascular-regular rate and rhythm, no murmers rubs or gallops
Gastrointestinal-normal active bowel sounds, soft, nontender nondistended, no appreciable hepatosplenomegaly or masses
Extremities 2+ bilateral lower extremity edema
Neurologic-moves all 4 extremities bilaterally, no facial droop

Skin-no lesions or rashes

**EKG-as described above **by my interpretation

[table#5]:
LABORATORY STUDIES

[table#6]:
Results from last 7 days
Lab	Units	12/23/21
0520	12/22/21
2027
WBC	K/mcL	8.0	9.2
HGB	g/dL	11.9	12.1
HEMATOCRIT	%	38.0	39.3
PLT	K/mcL	342	348
NEUTROS PCT AUTO	%	72.4	73.7
LYMPHS PCT AUTO	%	20.9	19.3
MONOS PCT AUTO	%	**4.0***	**4.2***
EOS PCT AUTO	%	1.8	1.5

[table#7]:
Results from last 7 days
Lab	Units	12/23/21
0520	12/22/21
2027
SODIUM	mmol/L	138	139
POTASSIUM	mmol/L	4.2	4.1
CHLORIDE	mmol/L	**97***	99
CO2	mmol/L	**36***	**35***
BUN	mg/dL	**31***	**28***
CREATININE	mg/dL	0.9	0.7
ANION GAP	mmol/L	**5***	**5***
CALCIUM	mg/dL	**8.2***	**8.2***
BILIRUBIN TOTAL	mg/dL	0.4	--
ALK PHOS	Units/L	83	--
ALT	Units/L	32	--
AST	Units/L	**38***	--
GLUCOSE	mg/dL	109	109

[table#8]:
| **Results from la
st 7 days** |
| --- |
| Lab | Units | 12/22/21
2027 |
| TROPONIN I | ng/mL | **0.035*** |

[table#9]:
POCT
Component	Value	Date
	COLORU	Yellow	12/22/2021
	CLARITYU	Clear	12/22/2021
	KETONESU	Negative	12/22/2021
	PROTEINUR	Negative	12/22/2021
	UROBILINOGEN	0.2	12/22/2021
	NITRITEU	Negative	12/22/2021

[table#10]:
| **Results from last 7 days** |

| --- |
| Lab | Units | 12/23/21
1341 |
PH ART		7.44
PCO2 ART	mm Hg	**59***
PO2 ART	mm Hg	**81***
HCO3 ART	mmol/L	**40.0***
O2 SAT ART	%	96.7
BASE EXC ART	mmol/L	**15.9***

[table#11]:
IMAGING

X-ray portable chest single frontal view

Result Date: 12/22/2021
IMPRESSION: Left midlung and retrocardiac opacities likely reflect atelectasis, although a developing infection cannot be completely excluded. Right lung is clear. No pulmonary edema. Moderate cardiomegaly. Small left pleural effusion. No pneumothorax.

Assessment
[table#12]:
ASSESSMENT AND PLAN

57 y.o.female with likely nonischemic cardiomyopathy diagnosed 1 year ago with a negative stress, prior methamphetamine abuse and medication nonadherence presents with dyspnea found to have an acute on chronic CHF exacerbation.

Active Problems:
Acute on chronic congestive heart failure (CMS/HCC)
Pulmonary edema
Asthma
Hypertension

#Acute on chronic nonischemic cardiomyopathy: Status post negative stress test 1 year ago. Endorsed on adherence to medications 1 year.
-Chest x-ray consistent with cardiomegaly, proBNP nearing 10,000.
-Echocardiogram TSH urine toxicology screen urinalysis, strict I's and O's Daily weights telemetry
-Restart lisinopril. Avoid restarting beta-blockade in the setting of acute CHF.
-Begin aspirin 81 mg.
-IV Lasix 40 mg twice daily
-Fluid restrict 1.2 L. Defer further troponin testing in the absence of chest pain or notable EKG changes

#Hypertensive emergency: This is improved somewhat with the first dose of Lasix.
-Restart lisinopril and IV Lasix
-Clonidine as needed as needed SBP 160
-Patient will need neuro humeral blockade check echocardiogram redemonstrated systolic failure.

#Mild macrocytosis: This may reflect her daily alcohol consumption. Check LFTs. No evidence of anemia, defer further work-up and.

#History of methamphetamine abuse: Patient currently denies. Check urine toxicology screen

#Asthma: No evidence of acute exacerbation clear lung fields aside from bibasilar rales.

Dispo; home/shelter pending arrangement of transportation to home/shelfter

Code Status: Full care full code

**DPOA: **
[table#13]:
Emergency Contacts
	No emergency contacts on file.

[table#14]:
| |

| --- |

Attending physician attestation:
I have reviewed this case with resident physician including:
1) History and Subjective findings.
2) Physical exam.
3) Laboratory results, X rays and other pertinent studies.

I discussed the assessment and plan with the resident physician.

57 y.o. female w/ Hx chronic heart failure (unknown EF) on furosemide 40 mg PO daily, HTN, and intermittent asthma admitted on 12/22/21 for decompensated heart failure.

She reports medication non-compliance. Her dyspnea on exertion improved s/p IV diuresis (UOP 1450 in 12 hours). She had no chest pain, dyspnea at rest, or ST changes on EKG to suggest ACS.

John Arcilla, M.D.
Internal Medicine

Electronically signed by John-Vincent Arcilla, MD at 12/23/2021 3:30 PM PST
Electronically signed by John-Vincent Arcilla, MD at 12/23/2021 3:30 PM PST

H&P Notes - documented in this encounter

Mark Lazarus, MD - 12/22/2021 10:20 PM PST
Formatting of this note might be different from the original.
[image]

Hospital Medicine History and Physical

PCP: No primary care provider on file.

[table#1]:
CHIEF COMPLAINT

Shortness of Breath (57-YR-OLD FEMALE WITH HX OF CHF BIBM FROM BOARDER WITH C/O SOB)

[table#2]:
HISTORY OF THE PRESENT ILLNESS

57 y.o.female history of cardiomyopathy notes 1 year ago in the setting of negative stress test, history of prior methamphetamine abuse, asthma who has been off of her medications since she lost her insurance 1 year ago presents with shortness of breath. Patient previously took carvedilol and lisinopril but is not taking those for nearly 1 year. She continues to take Lasix 40 mg daily but she does not have a primary care doctor or cardiologist. She lives in Mexico and gets her medications and that contract.

Over the past 2 weeks she endorses worsening shortness of breath dyspnea on exertion 4 pillow orthopnea and worsening pubic region lower extremity bilateral edema. She suspects that she is in congestive heart failure. Endorses not snorting methamphetamine for over a year. She drinks 1 alcoholic beverage daily but not to the point of intoxication does not have tremors or evidence of withdrawal.

Emergency department course notable for unremarkable BMP, troponin 0 0.035 proBNP 9890 and normal CBC with the exception of an MCV of 101. Influenza and Covid negative. Chest x-ray with cardiomegaly but otherwise unremarkable. KG was sinus tachycardia LVH left atrial large man

[table#3]:
REVIEW OF SYSTEMS

A complete review of systems was performed and is negative on a 12 point scale except as noted in HPI.

[table#4]:
ED COURSE

[table#5]:
Medications
nitroGLYcerin (NITRO-BID) 2 % ointment 1 inch (1 inch topical Medication Applied 12/22/21 2029)

| **lisinopriL (PRINIVIL) tablet 10 mg (has no administration in time range)** |
| **furosemide (LASIX) injection 40 mg (has no administration in time range)** |
| **aspirin EC tablet 81 mg (has no administration in time range)** |
| **cloNIDine (CATAPRES) tablet 0.1 mg (has no administration in time range)** |
| **enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg (has no administration in time range)** |
| **traZODone (DESYREL) tablet 50 mg (has no administration in time range)** |
| **acetaminophen (TYLENOL) tablet 650 mg (has no administration in time range)** |
| ** Or** |
| **acetaminophen (TYLENOL) suppository 650 mg (has no administration in time range)** |
| **ondansetron (PF) (ZOFRAN) injection 4 mg (has no administration in time range)** |
| **aspirin chewable tablet 162 mg (162 mg oral Given 12/22/21 2029)** |
| **furosemide (LASIX) injection 20 mg (20 mg intraVENOUS Given 12/22/21 2029)** |
| **metoCLOPramide (REGLAN) injection 10 mg (10 mg intraVENOUS Given 12/22/21 2213)** |

Objective
[table#6]:
PAST MEDICAL HISTORY

[table#7]:
Patient Active Problem List
Diagnosis
•	Acute on chronic congestive heart failure (CMS/HCC)

[table#8]:
Past Medical History:
Diagnosis	Date
•	Asthma	
•	CHF (congestive heart failure) (CMS/HCC)	

[table#9]:
PAST SURGICAL HISTORY

History reviewed. No pertinent surgical history.

[table#10]:
ALLERGIES

No Known Allergies

[table#11]:
MEDICATIONS

[table#12]:
**Prior to Admission medications **
Not on File

[table#13]:
FAMILY HISTORY

[table#14]:
Family History
Problem	Relation	Age of Onset
•	Hypertension	Father	

[table#15]:
SOCIAL HISTORY

[table#16]:

Social History

[table#17]:
Socioeconomic History
•	Marital status:	Not on file
		Spouse name:	Not on file
•	Number
of children:	Not on file
•	Years of education:	Not on file
•	Highest education level:	Not on file
Occupational History
•	Not on file
Tobacco Use
•	Smoking status:	Former Smoker
•	Smokeless tobacco:	Never Used
Substance and Sexual Activity
•	Alcohol use:	Not Currently
•	Drug use:	Not Currently
•	Sexual activity:	Not on file
Other Topics	Concern
•	Not on file
Social History Narrative
•	Not on file

[table#18]:
PHYSICAL EXAM

Vital Signs:
Temp: [37.2 °C (98.9 °F)] 37.2 °C (98.9 °F)
Heart Rate: [116-118] 117
Resp: [20-21] 21
BP: (131-154)/(100-113) 154/100
SpO2: 94 %

Physical Exam: All vital signs reviewed and O2 saturation by my interpretation

General- Alert and oriented x4/4 appears older than stated age anxious
HEENT-partially edentulous, normocephalic atraumatic, pupils equal round reactive sclera anicteric trachea midline, mucous membranes moist
Endocrine-no thyromegaly
Lymphatic-no significant lymphadenopathy
Lungs-decreased breath sounds with bibasilar rales present
Cardiovascular-11 cm JVD tachycardic rate and rhythm, no murmers rubs or gallops
Gastrointestinal-normal active bowel sounds, soft, nontender nondistended, no appreciable hepatosplenomegaly or masses
Extremities 2+ bilateral lower extremity edema
Neurologic-CN2-12 intact, remaining neuro exam is non-focal
Psychiatric-normal cognitive thought behavioral processes
Skin-no lesions or rashes

**EKG-as described above **by my interpretation

[table#19]:
LABORATORY STUDIES

[table#20]:
Results from last 7 days
Lab	Units	12/22/21
2027
WBC	K/mcL	9.2
HGB	g/dL	12.1
HEMATOCRIT	%	39.3
PLT	K/mcL	348
NEUTROS PCT AUTO	%	73.7
LYMPHS PCT AUTO	%	19.3
MONOS PCT AUTO	%	**4.2***

| EOS PCT AUTO | % | 1.5 |

[table#21]:
Results from last 7 days
Lab	Units	12/22/21
2027
SODIUM	mmol/L	139
POTASSIUM	mmol/L	4.1
CHLORIDE	mmol/L	99
CO2	mmol/L	**35***
BUN	mg/dL	**28***
CREATININE	mg/dL	0.7
ANION GAP	mmol/L	**5***
CALCIUM	mg/dL	**8.2***
GLUCOSE	mg/dL	109

[table#22]:
Results from last 7 days
Lab	Units	12/22/21
2027
TROPONIN I	ng/mL	**0.035***

No results found for: COLORU, CLARITYU, BILIRUBINUR, KETONESU, SPECGRAV, WBCU, RBCU, PHUR, PROTEINUR, UROBILINOGEN, NITRITEU

[table#23]:
IMAGING

X-ray portable chest single frontal view

Result Date: 12/22/2021
IMPRESSION: Left midlung and retrocardiac opacities likely reflect atelectasis, although a developing infection cannot be completely excluded. Right lung is clear. No pulmonary edema. Moderate cardiomegaly. Small left pleural effusion. No pneumothorax.

Assessment
[table#24]:
ASSESSMENT AND PLAN

57 y.o.female with likely nonischemic cardiomyopathy diagnosed 1 year ago with a negative stress, prior methamphetamine abuse and medication nonadherence presents with dyspnea found to have an acute on chronic CHF exacerbation.

Active Problems:
Acute on chronic congestive heart failure (CMS/HCC)

#Acute on chronic nonischemic cardiomyopathy: Status post negative stress test 1 year ago. Endorsed on adherence to medications 1 year.
-Chest x-ray consistent with cardiomegaly, proBNP nearing 10,000.
-Echocardiogram TSH urine toxicology screen urinalysis, strict I's and O's Daily weights telemetry
-Restart lisinopril. Avoid restarting beta-blockade in the setting of acute CHF.
-Begin aspirin 81 mg.
-IV Lasix 40 mg twice daily
-Fluid restrict 1.2 L. Defer further troponin testing in the absence of chest pain or notable EKG changes

#Hypertensive emergency: This is improved somewhat with the first dose of Lasix.
-Restart lisinopril and IV Lasix
-Clonidine as needed as needed SBP 160

-Patient will need neuro humeral blockade check echocardiogram redemonstrated systolic failure.

#Mild macrocytosis: This may reflect her daily alcohol consumption. Check LFTs. No evidence of anemia, defer further work-up and.

#History of methamphetamine abuse: Patient currently denies. Check urine toxicology screen

#Asthma: No evidence of acute exacerbation clear lung fields aside from bibasilar rales.

Code Status: Full care full code

**DPOA: **
[table#25]:
Emergency Contacts
	No emergency contacts on file.

[table#26]:

**DVT prophylaxis: **Lovenox

PLEASE NOTE THAT I DO NOT HAVE Epic HAIKU CHAT ON MY PHONE. PLEASE CALL ME 8am-10pm at 847.830.3636

I will take care of this patient until 2200 today. The night hospitalist will handle issues until 0800 tomorrow and the a.m. Brown team hospitalist will assume care of this patient following 0800 hrs tomorrow.

All the problems above are present on admission. The medical decision making process for this patient is complex, and the patient is at risk for morbidity and mortality.

[x]Complex MDM, patient is at [ ]high/[ ]moderate risk for morbidity or mortality due to the above.
[x]I have personally discussed with the [ ] primary team / [ ] other specialists:
[x] I spent 60 minutes on [x] direct care and review of relevant information.

Mark A Lazarus, MD

Scripps Mercy Chula Vista Hospitalist

Electronically signed by Mark Lazarus, MD at 12/22/2021 10:26 PM PST
Electronically signed by Mark Lazarus, MD at 12/22/2021 10:27 PM PST

ED Notes - documented in this encounter

Douglas Ballesteros, MD - 12/22/2021 7:23 PM PST
Formatting of this note might be different from the original.
[image]

**Name **Elizabeth Tina Petruccio
**DOB **7/29/1964
**Age **57 y.o.
**Chief Complaint: **
[table#1]:
Chief Complaint
Patient presents with
•	Shortness of Breath
		57-YR-OLD FEMALE WITH HX OF CHF BIBM FROM BOARDER WITH C/O SOB

**HPI: **
Elizabeth Tina Petruccio is a 57 y.o. female who presents with shortness of breath. Patient presents from across the border. She notes that she has been having progressively worsening shortness of breath and a sensation of fluid overload. About a year ago she was diagnosed with congestive heart failure. She has a known history of asthma and is a smoker. Denies any fevers or chills. Patient is otherwise been at her baseline. Due to worsening symptoms she came across the border for further evaluation. In route she did receive a breathing treatment. She does have improvement of symptoms after that breathing treatment although continues to have some mild shortness of breath.

Past Medical History

Patient has a past medical history of Asthma and CHF (congestive heart failure) (CMS/HCC).

Past Surgical History
has no past surgical history on file.

Meds:
[table#2]:
**Prior to Admission medications **
Not on File

Allergies: Patient has no known allergies.

Family History
Patient's family history includes Hypertension in her father.

Social History
Patient reports that she has quit smoking. She has never used smokeless tobacco. She reports previous alcohol use. She reports previous drug use.

**Review of Systems **
Comprehensive review of systems obtained and found to be negative other than mentioned in HPI.

VItal Signs:
[table#3]:
ED Triage Vitals
Temp	Pulse	Resp	BP	SpO2
--	**--**	**--**	**--**	**--**
Temp src	Heart Rate Source	Patient Position	BP Location	FiO2 (%)
--	**--**	**--**	**--**	**--**
** **
BP (!) 154/100 | Pulse 117 | Temp 37.2 °C (98.9 °F) (Oral) | Resp 21 | Ht 1.702 m (5' 7") | Wt 74.5 kg (164 lb 3.9 oz) | SpO2 94% | BMI 25.72 kg/m^2

Physical Exam. PPE utilized per current facility recommendation
Constitutional: well developed and nourished. non-toxic appearing.
Head: Normocephalic. Atraumatic.
Eyes: no scleral icterus. pupils round and reactive to light.
Ears: Hearing grossly intact. No traumatic lesions seen.
Nose: nose midline. No septal hematomas.
Oropharynx: Oropharynx is clear. No tonsillar hypertrophy. No bulging or erythema of the floor of the mouth or soft palate.
**Neck: **No midline bony step-off or crepitus. Range of motion intact.
Respiratory: no respiratory distress. Trace basilar crackles. Equal chest rise. No accessory muscle use. Speaking in full sentences.
Cardiovascular: normal rate. regular rhythm. no murmur. no cyanosis
Gastrointestinal: Abdomen is soft. Nondistended. Normoactive bowel sounds. No point area of tenderness.
Musculoskeletal: ranging neck normally. no traumatic lesions.
GU: deferred
Neurologic: Patient is awake and alert. No focal or lateralizing findings. Cranial nerves II through XII are grossly intact. No facial droop or asymmetry.
Skin: no jaundice or pallor. no petechia or purpura. No cyanosis
Psychiatric: calm. normal affect.

**Labs (all labs /ekg/ reviewed and interpreted by myself unless stated otherwise **
[table#4]:
Recent Results (from the past 24 hour(s))
12-Lead ECG
	Collection Time: 12/22/21 8:06 PM
Result	Value	Ref Range
	VENTRICULAR RATE EKG/MIN	113	bpm
	ATRIAL RATE	113	bpm
	PR-INTERVAL (MSEC)	146	ms
	QRS-INTERVAL (MSEC)	84	ms
	QT-INTERVAL (MSEC)	340	ms
	QTC	466	ms
	P AXIS	62	degrees
	R AXIS	40	degrees
	T AXIS

90 | degrees |
| **Basic metabolic panel** |
	Collection Time: 12/22/21 8:27 PM
Result	Value	Ref Range
	Fasting?		
	Sodium	139	137 - 145 mmol/L
	Potassium	4.1	3.5 - 5.1 mmol/L
	Chloride	99	98 - 107 mmol/L
	CO2, Total	35 (H)	22 - 30 mmol/L
	Anion Gap	5 (L)	6 - 14 mmol/L
	Urea Nitrogen	28 (H)	7 - 17 mg/dL
	Creatinine	0.7	0.5 - 1.0 mg/dL
	Glucose	109	70 - 125 mg/dL
	Osmolality Calculated	294	280 - 305 mOsm/kg H20
	Calcium	8.2 (L)	8.4 - 10.2 mg/dL
	GFR Calculation, Non-African American	>60	>60 mL/min/1.73m*2
	GFR Calculation, African American	>60	>60 mL/min/1.73m*2
Troponin I
	Collection Time: 12/22/21 8:27 PM
Result	Value	Ref Range
	Troponin I	0.035 (H)	<=0.034 ng/mL
NT Pro-BNP
	Collection Time: 12/22/21 8:27 PM
Result	Value	Ref Range
	NT pro-BNP	9,890.0 (H)	<=125.0 pg/mL
CBC with Differential
	Collection Time: 12/22/21 8:27 PM
Result	Value	Ref Range
	WBC	9.2	3.4 - 11.0 K/mcL
	RBC	3.91 (L)	3.98 - 5.25 M/mcL
	HGB	12.1	11.9 - 15.3 g/dL
	HCT	39.3	37.3 - 46.7 %
	MCV	101 (H)	81 - 100 fL
	MCH	31	26 - 33 pg
	MCHC	30.8 (L)	31.0 - 36.0 g/dL
	RDW	13.5	11.7 - 14.9 %
	Platelet Count	348	150 - 425 K/mcL
	nRBC	0.0	0 /100 WBCs
	Preliminary Abs Neut	6.74	1.50 - 7.40 K/mcL
	Neutrophils %	73.7	38.0 - 74.0 %
	IG%	0.8	0.0 - 1.2 %
	Lymphocytes %	19.3	16.0 - 48.0 %
	Monocytes %	4.2 (L)	4.9 - 12.5 %
	Eosinophils %	1.5	0.0 - 9.5 %
	Basophils %	0.5	0.0 - 1.6 %
	Absolute Neutrophils	6.74	1.50 - 7.40 K/mcL
	IG#	0.07	0.00 - 0.10 K/mcL
	Absolute Lymphocytes	1.77	0.90 - 3.10 K/mcL
	Absolute Monocytes	0.38	0.26 - 0.87 K/mcL
	Absolute Eosinophils	0.14	0.00 - 0.51 K/mcL
	Absolute Basophils	0.05	0.00 - 0.09 K/mcL
Influenza A/B
	Collection Time: 12/22/21 9:08 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	Influenza A PCR	Not Detected	Not Detected
	Influenza B PCR	Not Detected	Not Detected
COVID-19, Rapid (PCR/NAAT COMBO)
	Collection Time: 12/22/21 9:08 PM
	Specimen: Nasopharyngeal; Swab
Result	Value	Ref Range
	COVID-19, Rapid (PCR/NAAT)	Not Detected	Not Detected

**Imaging:(all imaging studies reviewed and interpreted by myself unless stated otherwise **
[table#5]:
X-ray portable chest single frontal view
Final Result
IMPRESSION:

| Left midlung and retrocardiac opacities likely reflect atelectasis, although a developing infection cannot be completely excluded. Right lung is clear. No pulmonary edema. |
| |
| Moderate cardiomegaly. |
| |
| Small left pleural effusion. No pneumothorax. |
| |
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| |

** ED COURSE:**

Patient was evaluated not have a history concerning for acute CHF exacerbation. As mentioned she been off of her medications for some time. Fortunately the Covid test returned negative and there is no signs of any pneumonia. Her cardiac enzymes did return elevated both the BNP and troponin. This along with her history goes along with an acute CHF exacerbation. Relatively mild elevation in the troponin is more likely strain from the CHF. Plan is for admission for further evaluation of the CHF and chest pain. Patient is stable at time of admission to the telemetry unit.

[table#6]:
ED Course as of 12/22/21 2209
Wed Dec 22, 2021
2142	**12-Lead ECG**
Sinus tachycardia at a rate of 113. PR interval 146. QT corrected 466. LVH no
ted. No acute ST elevations. No old EKG for comparison. [DB]
2142	**Troponin I(!): 0.035** [DB]
2142	**NT pro-BNP(!): 9,890.0** [DB]
2142	**COVID-19, Rapid (PCR/NAAT): Not Detected** [DB]
ED Course User Index
[DB] Douglas Anthony Ballesteros, MD

ED Meds:
[table#7]:
Medications
nitroGLYcerin (NITRO-BID) 2 % ointment 1 inch (1 inch topical Medication Applied 12/22/21 2029)
metoCLOPramide (REGLAN) injection 10 mg (has no administration in time range)
aspirin chewable tablet 162 mg (162 mg oral Given 12/22/21 2029)
furosemide (LASIX) injection 20 mg (20 mg intraVENOUS Given 12/22/21 2029)

Procedures

Clinical Impression:
[table#8]:
1.	**Acute on chronic congestive heart failure, unspecified heart failure type (CMS/HCC) **
2.	Chest pain, unspecified type Acute

Follow-up:
No follow-up provider specified.

Disposition:

Medications and Referrals:
[table#9]:
ED Prescriptions
	None

This plan was made during the Covid-19 pandemic. As such, it may have deviated from usual medical decision making due to current resource allocations and/or to minimize the patients's contacts with the healthcare system to prevent direct exposures and

nosocomial transmission.

Part of this note was completed using Nuance dictation system which could lead to errors during speech to text transcription.

Douglas Anthony Ballesteros, MD
12/22/21 2209

Electronically signed by Douglas Ballesteros, MD at 12/22/2021 10:09 PM PST

Plan of Treatment - documented as of this encounter

Scheduled Orders

Name	Type	Priority	Associated Diagnoses	Order Schedule
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)	Cardiac Services			Ordered: 12/22/2021
CARDIOLOGY EXTERNAL REPORT (CARDIAC SERVICES)	Cardiac Services			Ordered: 12/22/2021

Health Maintenance	Due Date	Last Done	Comments
Pneumococcal 50+ (1 of 2 - PCV)	**07/29/1983**		
Breast Cancer Screening	**07/29/2004**		
Colorectal Cancer Screening	**07/29/2009**		
RSV (1 - Risk 50-74 years 1-dose series)	**07/29/2014**		
Shingrix Vaccine (1 of 2)	**07/29/2014**		
Cervical Cancer Screening	**02/27/2024**	02/27/2019, 01/20/2012	
Flu Shot (#1)	**09/01/2026**	11/20/2012	
Adult Tdap/Td Vaccines (2 - Td or Tdap)	02/09/2034	02/09/2024	

Results - documented in this encounter

BLOOD GAS, ARTERIAL - Final result (12/23/2021 1:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
pH, Arterial	7.44	7.35 - 7.45		12/23/2021 1:47 PM PST	MERCY LAB CV	
pCO2, Arterial	59	35 - 45 mm Hg		12/23/2021 1:47 PM PST	MERCY LAB CV	
pO2, Arterial	81	83 - 108 mm Hg		12/23/2021 1:47 PM PST	MERCY LAB CV	
HCO3, Arterial	40.0	21.0 - 28.0 mmol/L		12/23/2021 1:47 PM PST	MERCY LAB CV	
O2 Sat, Arterial	96.7	95.0 - 98.0 %		12/23/2021 1:47 PM PST	MERCY LAB CV	
Base Excess/Deficit, Arterial	15.9	-2.0 - 3.0 mmol/L		12/23/2021 1:47 PM PST	MERCY LAB CV	
Oxygen Device	Room Air			12/23/2021 1:47 PM PST	MERCY LAB CV	
Modes	NA			12/23/2021 1:47 PM PST	MERCY LAB CV	
Notified By	243446			12/23/2021 1:47 PM PST	MERCY LAB CV	
Notified Whom	166427			12/23/2021 1:47 PM PST	MERCY LAB CV	
Operator ID	243,446			12/23/2021 1:47 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Arterial blood / Unknown	Arterial Puncture / Unknown	12/23/2021 1:41 PM PST	12/23/2021 1:47 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
John-Vincent Arcilla MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (12/23/2021 5:20 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	8.0	3.4 - 11.0 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
RBC	3.82	3.98 - 5.25 M/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HGB	11.9	11.9 - 15.3 g/dL		12/23/2021 8:07 AM PST	MERCY LAB CV	
HCT	38.0	37.3 - 46.7 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
MCV	100	81 - 100 fL		12/23/2021 8:07 AM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		12/23/2021 8:07 AM PST	MERCY LAB CV	
MCHC	31.3	31.0 - 36.0 g/dL		12/23/2021 8:07 AM PST	MERCY LAB CV	
RDW	13.8	11.7 - 14.9 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Platelet Count	342	150 - 425 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
nRBC	0.0	0 /100 WBCs		12/23/2021 8:07 AM PST	MERCY LAB CV	
Preliminary Abs Neut	5.80	1.50 - 7.40 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Neutrophils %	72.4	38.0 - 74.0 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
IG%	0.4	0.0 - 1.2 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Lymphocytes %	20.9	16.0 - 48.0 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Monocytes %	4.0	4.9 - 12.5 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Eosinophils %	1.8	0.0 - 9.5 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Basophils %	0.5	0.0 - 1.6 %		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Neutrophils	5.80	1.50 - 7.40 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
IG#	0.03	0.00 - 0.10 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Lymphocytes	1.67	0.90 - 3.10 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Monocytes	0.32	0.26 - 0.87 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	
Absolute Eosinophils	0.14	0.00 - 0.51 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Basophils	0.04	0.00 - 0.09 K/mcL		12/23/2021 8:07 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/23/2021 5:20 AM PST	12/23/2021 7:54 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
John-Vincent Arcilla MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COMPREHENSIVE METABOLIC PANEL - Final result (12/23/2021 5:20 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/23/2021 6:13 AM PST	MERCY LAB CV	
Sodium	138	137 - 145 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
Potassium	4.2	3.5 - 5.1 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
Chloride	97	98 - 107 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
CO2, Total	36	22 - 30 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
Glucose	109	70 - 125 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Urea Nitrogen	31	7 - 17 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Creatinine	0.9	0.5 - 1.0 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Osmolality Calculated	293	280 - 305 mOsm/kg H20		12/23/2021 6:13 AM PST	MERCY LAB CV	
Albumin	3.1	3.5 - 5.0 g/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Protein, total	6.1	6.3 - 8.2 g/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Alkaline Phosphatase	83	38 - 126 Units/L		12/23/2021 6:13 AM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
AST	38	14 - 36 Units/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
Bilirubin, Total	0.4	0.2 - 1.3 mg/dL		12/23/2021 6:13 AM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
ALT	32	<=34 Units/L		12/23/2021 6:13 AM PST	MERCY LAB CV	
GFR Calculation, African American	>60	>60 mL/ min/ 1.73m*2		12/23/2021 6:13 AM PST	MERCY LAB CV	
GFR Calculation, Non-African American	>60	>60 mL/ min/ 1.73m*2		12/23/2021 6:13 AM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/23/2021 5:20 AM PST	12/23/2021 5:42 AM PST

Narrative

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MERCY LAB CV - 12/23/2021 6:13 AM PST
eGFR not calculated on patients <18 years of age.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:       Normal
100-125 mg/dL:     Impaired Fasting Glucose
>125 mg/dL:        Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

URINALYSIS DIPSTICK W/ REFLEX TO MICROSCOPIC (URINALYSIS WITH MICROSCOPIC IF INDICATED) - Final result (12/22/2021 10:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Color (UA)	Yellow	Yellow		12/22/2021 10:46 PM PST	MERCY LAB CV	
Clarity (UA)	Clear	Clear		12/22/2021 10:46 PM PST	MERCY LAB CV	
Specific Gravity (UA)	1.010	1.005 - 1.030		12/22/2021 10:46 PM PST	MERCY LAB CV	
pH (UA)	6.0	5.0 - 8.5 pH		12/22/2021 10:46 PM PST	MERCY LAB CV	
Protein (UA)	Negative	Negative mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Glucose (UA)	Negative	Negative mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	
Ketones (UA)	Negative	Negative mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	
Bilirubin (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Occult Blood (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Leukocyte Esterase (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Nitrite (UA)	Negative	Negative		12/22/2021 10:46 PM PST	MERCY LAB CV	
Urobilinogen (UA)	0.2	<2.0 mg/dL		12/22/2021 10:46 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen collection, clean catch / Unknown		12/22/2021 10:41 PM PST	12/22/2021 10:43 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

RAPID DRUG SCREEN, URINE (DRUG SCREEN, URINE) - Final result (12/22/2021 10:41 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Benzodiazepines (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Cocaine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 150						
Tetrahydrocannabinol (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 50						
Opiates (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						
Methamphetamine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Barbiturates (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Amphetamine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 500						
Methadone (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 200						
Phencyclidine (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 25						
Tricyclic Antidepressants (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 300						
Oxycodone (UR)	Negative	Negative ng/mL		12/22/2021 11:04 PM PST	MERCY LAB CV	
Comment: Negative Result Threshold Less Than 100						

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Urine	Urine specimen collection, clean catch / Unknown		12/22/2021 10:41 PM PST	12/22/2021 10:43 PM PST

Narrative

MERCY LAB CV - 12/22/2021 11:04 PM PST
This method provides screening results for medical purposes only. A more specific alternate chemical method must be used for a confirmed analytical result. Clinical considerations and professional judgement should be applied to any drug of abuse result, particularly when screening positive results are used.

Contact laboratory to order confirmatory testing.

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB URINE ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

COVID-19, RAPID COMBO (COVID-19, RAPID (PCR/NAAT, COMBO)) - Final result (12/22/2021 9:08 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
COVID-19, Rapid (PCR/ NAAT)	Not Detected	Not Detected		12/22/2021 9:37 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

A Detected result is considered a positive result for that virus.
A positive SARS-CoV-2 result indicates that RNA from SARS-CoV-2 was detected and that the patient is considered infected and presumed contagious. A positive Influenza A or B result indicates that RNA from Influenza A or B, respectively, was detected and that the patient is considered infected and presumed contagious.

A Not Detected (negative) test result means that RNA was not present in the specimen above the limit of detection. However, it does not rule out the possibility of infection and should not be used as the sole basis for patient management decisions. Test parameters for asymptomatic patients have not been evaluated.

These results have been reported to the San Diego Dept. of Health Services as required by Title 17, California Code of regulations, Section 2500.
Test information for patients can be found at the following link: https://www.fda.gov/media/142192/download
Test information for providers can be found at the following link: https://www.fda.gov/media/142191/download

This SARS-CoV-2 Real Time RT-PCR test was performed under an Emergency Use Authorization granted by the FDA for the Roche Liat.

This test meets Covid-19 testing requirements for school and business testing criteria and is not an antibody or antigen test.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		12/22/2021 9:08 PM PST	12/22/2021 9:11 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

INFLUENZA A/B COMBO - Final result (12/22/2021 9:08 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Influenza A PCR	Not Detected	Not Detected		12/22/2021 9:37 PM PST	MERCY LAB CV	
Influenza B PCR	Not Detected	Not Detected		12/22/2021 9:37 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Swab	Nasopharyngeal structure / Unknown		12/22/2021 9:08 PM PST	12/22/2021 9:11 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

X-ray portable chest single frontal view (X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW) - Final result (12/22/2021 8:29 PM PST)

Anatomical Region	Laterality	Modality
Body		Computed Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/22/2021 8:37 PM PST	

Impressions

12/22/2021 8:39 PM PST
IMPRESSION:

Left midlung and retrocardiac opacities likely reflect atelectasis, although a developing infection cannot be completely excluded. Right lung is clear. No pulmonary edema.

Moderate cardiomegaly.

Small left pleural effusion. No pneumothorax.

Narrative

12/22/2021 8:39 PM PST
EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/22/2021 8:29 PM

HISTORY: chest pain

COMPARISON: None.

FINDINGS:

See impression.

Procedure Note

Christopher Maxwell, MD - 12/22/2021

Procedure Note

Formatting of this note might be different from the original.

EXAMINATION: X-RAY PORTABLE CHEST SINGLE FRONTAL VIEW

DATE/TIME: 12/22/2021 8:29 PM

HISTORY: chest pain

COMPARISON: None.

FINDINGS:

See impression.

IMPRESSION:
IMPRESSION:

Left midlung and retrocardiac opacities likely reflect atelectasis, although a developing infection cannot be completely excluded. Right lung is clear. No pulmonary edema.

Moderate cardiomegaly.

Small left pleural effusion. No pneumothorax.

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	IMG DIAGNOSTIC IMAGING PROCEDURES	Final Result

LIPID PANEL (LIPID PANEL WITH CALC LDL) - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/23/2021 10:28 AM PST	SCRIPPS LAB	
Triglycerides	59	<150 mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	
Comment:						
Normal <150 mg/dL Borderline High 150-199 mg/dL High 200-499 mg/dL Very high >499 mg/dL						
Cholesterol	122	<200 mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	
Comment:						
Desirable <200 mg/dL Borderline High 200-239 mg/dL High >239 mg/dL						
HDL Cholesterol	62	Not Established mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Comment:						
Low <40 mg/dL High >59 mg/dL(Negative Risk Factor)						
LDL Calculation	48	<130 mg/dL		12/23/2021 10:28 AM PST	SCRIPPS LAB	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

SCRIPPS LAB - 12/23/2021 10:28 AM PST
Reference ranges are derived from the clinical practice ATPIII Guidelines of the National Cholesterol Education Program (NCEP) for the prevention and management of high cholesterol in adults (September 2002).

The relationship between levels of LDL cholesterol and Coronary Heart Disease is continuous over a broad range of LDL levels. A reduction in Cholesterol, Triglycerides and LDL levels has been associated with a reduced risk of CHD.

http://www.nhlbi.nih.gov/guidelines/cholesterol/index.htm

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS LAB	9535 Waples St.,150	San Diego, CA 92121	858-554-9552

TSH (THYROID STIMULATING HORMONE) - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Thyroid Stimulating Hormone	0.679	0.465 - 4.680 mcIU/mL		12/23/2021 2:20 AM PST	MERCY LAB CV	
Comment: High dose biotin supplementation (>1 mg/day) may falsely decrease results.						

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Mark Lazarus MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

CBC WITH DIFFERENTIAL - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	9.2	3.4 - 11.0 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
RBC	3.91	3.98 - 5.25 M/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
HGB	12.1	11.9 - 15.3 g/dL		12/22/2021 8:54 PM PST	MERCY LAB CV	
HCT	39.3	37.3 - 46.7 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
MCV	101	81 - 100 fL		12/22/2021 8:54 PM PST	MERCY LAB CV	
MCH	31	26 - 33 pg		12/22/2021 8:54 PM PST	MERCY LAB CV	
MCHC	30.8	31.0 - 36.0 g/dL		12/22/2021 8:54 PM PST	MERCY LAB CV	
RDW	13.5	11.7 - 14.9 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Platelet Count	348	150 - 425 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
nRBC	0.0	0 /100 WBCs		12/22/2021 8:54 PM PST	MERCY LAB CV	
Preliminary Abs Neut	6.74	1.50 - 7.40 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Neutrophils %	73.7	38.0 - 74.0 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
IG%	0.8	0.0 - 1.2 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Lymphocytes %	19.3	16.0 - 48.0 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Monocytes %	4.2	4.9 - 12.5 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Eosinophils %	1.5	0.0 - 9.5 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Basophils %	0.5	0.0 - 1.6 %		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Neutrophils	6.74	1.50 - 7.40 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
IG#	0.07	0.00 - 0.10 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Lymphocytes	1.77	0.90 - 3.10 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Monocytes	0.38	0.26 - 0.87 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Eosinophils	0.14	0.00 - 0.51 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	
Absolute Basophils	0.05	0.00 - 0.09 K/mcL		12/22/2021 8:54 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

N-TERMINAL PROBNP (NT PRO-BNP) - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
NT pro-BNP	9,890.0	<=125.0 pg/mL		12/22/2021 9:19 PM PST	MERCY LAB CV	

Comment:

```
Optimal Cut-Points (pg/mL)
<50 years old:  <= 450
50-75 years old: <= 900
>75 years old: <= 1800
Reference: Januzzi JL et. al.
European Heart Journal. 2006, 27: 330-337

High dose biotin supplementation (>1 mg/day) may falsely decrease results.
```

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

TROPONIN I - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	0.035	<=0.034 ng/mL		12/22/2021 9:19 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature

Comment:

The decision limit value of 0.035 ng/mL is based on the established 99th percentile upper reference limit in a healthy reference population. This patient's result is suspicious for myocardial injury. However, serial measurements may be necessary to confirm or exclude the diagnosis of acute myocardial infarction.

High dose biotin supplementation (>1 mg/day) may falsely decrease results.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

BASIC METABOLIC PANEL - Final result (12/22/2021 8:27 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Fasting?				12/22/2021 9:07 PM PST	MERCY LAB CV	
Sodium	139	137 - 145 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Potassium	4.1	3.5 - 5.1 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Chloride	99	98 - 107 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
CO2, Total	35	22 - 30 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Anion Gap	5	6 - 14 mmol/L		12/22/2021 9:07 PM PST	MERCY LAB CV	
Urea Nitrogen	28	7 - 17 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
Creatinine	0.7	0.5 - 1.0 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
Glucose	109	70 - 125 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	
Osmolality Calculated	294	280 - 305 mOsm/kg H20		12/22/2021 9:07 PM PST	MERCY LAB CV	
Calcium	8.2	8.4 - 10.2 mg/dL		12/22/2021 9:07 PM PST	MERCY LAB CV	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
GFR Calculation, Non-African American	>60	>60 mL/min/1.73m*2		12/22/2021 9:07 PM PST	MERCY LAB CV	
GFR Calculation, African American	>60	>60 mL/min/1.73m*2		12/22/2021 9:07 PM PST	MERCY LAB CV	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Venous blood / Unknown	Venipuncture / Unknown	12/22/2021 8:27 PM PST	12/22/2021 8:45 PM PST

Narrative

```
MERCY LAB CV - 12/22/2021 9:07 PM PST
eGFR not calculated on patients <18 years of age.
For FASTING GLUCOSE samples, the ADA recommended decision limits are shown below:
70-99 mg/dL:        Normal
100-125 mg/dL:      Impaired Fasting Glucose
>125 mg/dL:         Diabetes*

*In the absence of symptoms of unequivocal hyperglycemia, result should be confirmed by repeat testing.
```

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
MERCY LAB CV	435 H. Street	Chula Vista, CA 91910	619-691-7284

12-LEAD ECG - Final result (12/22/2021 8:06 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG/MIN	113	bpm			SCRIPPS RADIOLOGY	
ATRIAL RATE	113	bpm			SCRIPPS RADIOLOGY	
PR-INTERVAL (MSEC)	146	ms			SCRIPPS RADIOLOGY	
QRS-INTERVAL (MSEC)	84	ms			SCRIPPS RADIOLOGY	
QT-INTERVAL (MSEC)	340	ms			SCRIPPS RADIOLOGY	
QTC	466	ms			SCRIPPS RADIOLOGY	
P AXIS	62	degrees			SCRIPPS RADIOLOGY	
R AXIS	40	degrees			SCRIPPS RADIOLOGY	
T AXIS	90	degrees			SCRIPPS RADIOLOGY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/22/2021 8:06 PM PST	12/23/2021 7:27 AM PST

Impressions

```
SCRIPPS RADIOLOGY - 12/23/2021 7:28 AM PST
Sinus tachycardia
Possible Left atrial enlargement
Left ventricular hypertrophy with repolarization abnormality ( Cornell product )
Abnormal ECG
No previous ECGs available
```

Narrative

This result has an attachment that is not available.

Procedure Note

Douglas Ballesteros, MD - 12/23/2021

Formatting of this note might be different from the original.

IMPRESSION:
Sinus tachycardia
Possible Left atrial enlargement
Left ventricular hypertrophy with repolarization abnormality (Cornell product)
Abnormal ECG
No previous ECGs available

Authorizing Provider	Result Type	Result Status
Douglas Ballesteros MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
SCRIPPS RADIOLOGY		US	

Visit Diagnoses - documented in this encounter

Diagnosis
Heart failure - Primary
Chest pain, unspecified type
Heart failure
Fluid in the lungs
Asthma
High blood pressure

Administered Medications - documented in this encounter

Inactive Administered Medications - up to 3 most recent administrations

Medication Order	MAR Action	Action Date	Dose	Rate	Site
acetaminophen (TYLENOL) suppository 650 mg 650 mg, rectal, Every 6 hours PRN, for, temperature greater than 38 C (100.4 F), Starting on Thu 12/23/21 at 0537, Do not exceed 4 g Acetaminophen per day `					
acetaminophen (TYLENOL) tablet 650 mg 650 mg, oral, Every 6 hours PRN, for, temperature greater than 38 C (100.4 F), Starting on Wed 12/22/21 at 2220, Do not exceed 4 g Acetaminophen per day `	Given	12/22/2021 11:45 PM PST	650 mg		
acetaminophen (TYLENOL) tablet 650 mg 650 mg, oral, Every 6 hours PRN, moderate pain (level 4-6), temperature greater than 38.6 C (101.5 F), for, temperature greater than 38 C (100.4 F), Starting on Thu 12/23/21 at 0537, Do not exceed 4 g Acetaminophen per day `	Given Given	12/23/2021 2:08 PM PST 12/23/2021 5:47 AM PST	650 mg 650 mg		
albuterol 2.5 mg /3 mL (0.083 %) nebulizer solution 2.5 mg 2.5 mg, nebulization, Every 2 hours PRN, wheezing, Starting on Thu 12/23/21 at 1114, `					
aspirin chewable tablet 162 mg 162 mg, oral, Once, On Wed 12/22/21 at 2030, For 1 dose, If no contraindication and not taken prior to arrival `	Given	12/22/2021 8:29 PM PST	162 mg		
aspirin EC tablet 81 mg 81 mg, oral, Daily, First dose on Thu 12/23/21 at 0900, `Do Not Crush	Given	12/23/2021 7:59 AM PST	81 mg		
benzonatate (TESSALON) capsule 100 mg 100 mg, oral, 3 times daily PRN, cough, Starting on Thu 12/23/21 at 1323, `Do not crush, chew, break or open. Swallow whole.	Given	12/23/2021 1:43 PM PST	100 mg		
carvediloL (COREG) tablet 3.125 mg 3.125 mg, oral, 2 times daily, First dose on Thu 12/23/21 at 1130, `	Given	12/23/2021 1:43 PM PST	3.125 mg		
cloNIDine (CATAPRES) tablet 0.1 mg 0.1 mg, oral, 2 times daily PRN, SBP>160, Starting on Wed 12/22/21 at 2219, hold for SBP under 100 or HR less than 60 `					
enoxaparin (LOVENOX) 40 mg/0.4 mL syringe 40 mg 40 mg, subcutaneous, Daily, First dose on Wed 12/22/21 at 2245, `	Given Given	12/23/2021 8:01 AM PST 12/22/2021 10:48 PM PST	40 mg 40 mg		Left Lower Abdomen Right Lower Abdomen
furosemide (LASIX) injection 20 mg 20 mg, intraVENOUS, Once, On Wed 12/22/21 at 2030, For 1 dose, `Do not refrigerate	Given	12/22/2021 8:29 PM PST	20 mg		
furosemide (LASIX) injection 40 mg 40 mg, intraVENOUS, Every 12 hours scheduled, First dose on Thu 12/23/21 at 0800, `Do not refrigerate	Given	12/23/2021 7:59 AM PST	40 mg		

Medication Order	MAR Action	Action Date	Dose	Rate	Site
lisinopriL (PRINIVIL) tablet 10 mg 10 mg, oral, Daily, First dose on Wed 12/22/21 at 2245, `	Given Given	12/23/2021 7:59 AM PST 12/22/2021 10:48 PM PST	10 mg 10 mg		
metoCLOPramide (REGLAN) injection 10 mg 10 mg, intraVENOUS, Once, On Wed 12/22/21 at 2230, For 1 dose, For IVP: slowly over 2 min. `	Given	12/22/2021 10:13 PM PST	10 mg		
multivit (Thera M Plus), iron 9 mg, folic acid 400 mcg, calcium and minerals tablet 1 tablet, oral, Daily, First dose on Thu 12/23/21 at 1130, `	Given	12/23/2021 1:43 PM PST	1 tablet		
nitroGLYcerin (NITRO-BID) 2 % ointment 1 inch 1 inch, topical, Administer over 6 Hours, Once PRN, chest pain, if SBP is greater than 100 mmHg, Starting on Wed 12/22/21 at 2002, For 1 dose, Apply to chest `	Medication Applied	12/22/2021 8:29 PM PST	1 inch		Chest
ondansetron (PF) (ZOFRAN) injection 4 mg 4 mg, intraVENOUS, Every 6 hours PRN, nausea or vomiting, Starting on Wed 12/22/21 at 2220, Individual doses of 8 mg or less may be given as IV push over 2-3 minutes `					
traMADoL (ULTRAM) tablet 25 mg 25 mg, oral, Every 6 hours PRN, moderate pain (level 4-6), Starting on Thu 12/23/21 at 0110, `	Given Given	12/23/2021 10:50 AM PST 12/23/2021 1:25 AM PST	25 mg 25 mg		
traZODone (DESYREL) tablet 50 mg 50 mg, oral, At bedtime, First dose on Wed 12/22/21 at 2345, `	Given	12/22/2021 11:45 PM PST	50 mg		

Insurance - documented as of this encounter

Guarantor: Petruccio, Elizabeth Tina

Account Type	Relation to Patient	Date of Birth	Phone	Billing Address
Personal/Family	Self	07/29/1964		HOMELESS SAN DIEGO, CA 99999

CALOPTIMA MEDI-CAL

Member	Subscriber	Plan / Payer (Effective 01/01/2021-01/11/2024)
Name: Petruccio, Elizabeth Tina Member ID: 93931022D Relation to Subscriber: Self	Name: Petruccio, Elizabeth Tina Subscriber ID: 93931022D	Payer ID: Not on file Group ID: Not on file Type: Medi-Cal HMO Phone: 714-246-8686 Address: PO BOX 11037 ORANGE, CA 92856-8137

Advance Directives

For more information, please contact: 858-927-7132 (6AM - 4PM America/Los_Angeles, Monday-Friday)

Full Code (Latest Code Status on File)

Date Activated	Date Inactivated	Comments
1/12/2024 9:06 PM	2/17/2024 6:01 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:20 PM	12/23/2021 8:29 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Full Code

Date Activated	Date Inactivated	Comments
12/22/2021 10:09 PM	12/22/2021 10:20 PM	

Question	Answer	Comments
Mechanical Ventilation with Intubation:	Yes	
Care level:	Standard Care	
Transfer to critical care:	Yes	
Non-invasive ventilation:	Yes	
Dialysis:	Yes	
Blood or blood products:	Yes	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Erika Dimarello	Unknown	609-802-3373 (Mobile)	Daughter, Personal Relationship
Anthony Dimarello	Unknown	949-556-1803 (Mobile)	Son, Personal Relationship

Document Information

Service Providers

Document Coverage Dates

Dec. 22, 2021 - Dec. 23, 2021

Custodian Organization

Scripps Health
858-927-7132 (Work)
4275 Campus Point Court
SAN DIEGO, CA 92121

Encounter Providers

Encounter Date

Dec. 22, 2021 - Dec. 23, 2021

Douglas Ballesteros, MD (Attending)
NPI: 1275572372
619-691-7290 (Work)
619-691-7435 (Fax)
435 H St
Chula Vista, CA 91910-4307
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